Harmonia

Enterprise Architecture Capability Map

Harmonia-EA-Capability-Map
Version 0.2-draft

Proposed Complete Architecture

Human-reviewed BEC Reconciliation
16 October 2026
`Harmonia-GovernedAccess`

Preserved Canonical semantics and identities with the reviewed candidate BEC hierarchy.

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Part I.

Publication guide

1. Reading the proposed architecture

Version 0.2-draft incorporates the human-reviewed BEC candidate hierarchy and a bounded reconciliation of compatible Stage-2 material. It does not replace the governed Markdown RAD or declare an architecture domain complete. The 0.1 publication remains independently reproducible. Its identities, definitions, semantic boundaries, source traceability and established relationships are preserved, with genuine conflicts explicitly retained outside the current BEC hierarchy.

Element, naming, direct-placement, complete-path and relationship status are separate assertions. A Canonical responsibility can have a Proposed CT3 placement and Proposed structural identifier. A Candidate concern remains Candidate even when a best placement is proposed. Source uncertainty is retained in the governed appendix; the proposal makes a current judgement rather than leaving the concept floating.

Capability, Feature, Function, Service and Process remain different constructs. Every current Feature has a meaningful BEC CT3 frame. Features whose original semantics have no adjudicated frame are preserved in the explicit unresolved register. Directly Capability-owned behaviour remains valid. Services expose a client contract; Processes chain Functions where meaningful. None of those semantics requires a Feature-to-Function-to-Service-to-Process containment chain.

The fourteen Canonical EC responsibilities become CT2s under three Proposed CT1 frames. The application of the structural ID grammar to ECs is itself Proposed; EC-01 through EC-14 remain stable Reference IDs and historical structural aliases. Reuse does not mandate centralised services, separate processes or distributed responsibility.

Downstream Application Component derivation may use this current proposed model as its working assumption, while retaining every assertion status. Derive cohesive responsibility centres from capabilities; do not force them to reproduce current Mneme, Mnemosyne, Ponos, Calliope or other component boundaries. No capability in this publication allocates an application component, module, process or deployment.

AX-01/04 preserve healthcare meaning over machinery; AX-05 separates active use from durable establishment; AX-06 qualifies source authority; AX-07/08/09 distinguish governed action, meaningful evidence and diagnostics; AX-10/11/12 preserve operational integrity; AX-13 governs boundary publication; AX-14/15/17 preserve distinctions, uncertainty and authority; AX-16/18 preserve entity progression without inferred domain authority. CM-R01 through CM-R05 are reviewed through complete placement, bounded responsibility, explicit definitions, cohesive roots and unique current display names.

Standing	Elements
Canonical	716
Candidate	3
Proposed	227

The human-reviewed reconciliation register records surviving responsibilities, scope and HUMAN ADJUDICATION REQUIRED items. Earlier Step-2 review questions are historical evidence; they do not override the reviewed hierarchy.

Part II.

Business Capability Architecture

The established 190-node BC hierarchy is retained. Six Proposed CT3 refinements and one Proposed preparedness CT2 are additive. The three existing Candidate concerns retain Candidate status and receive Proposed placement. Clinical and enterprise authority remain separate from Harmonia information support.

CT1 / page	**Element status**	**Placement**
BC CT1: Individual Care Delivery (p. 5)	Canonical	Established
BC CT1: Care Access & Coordination (p. 26)	Canonical	Established
BC CT1: Health Rights, Advocacy & Participation (p. 48)	Canonical	Established
BC CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 50)	Canonical	Established
BC CT1: Health Products & Clinical Technology (p. 65)	Canonical	Established
BC CT1: Clinical Quality, Safety & Improvement (p. 75)	Canonical	Established
BC CT1: Community Health & Wellbeing (p. 85)	Canonical	Established
BC CT1: Population Health & Health-System Planning (p. 93)	Canonical	Established
BC CT1: Health Information & Knowledge Management (p. 104)	Canonical	Established
BC CT1: Standards, Semantics & Reference Governance (p. 120)	Canonical	Established
BC CT1: Connected Health Services (p. 135)	Canonical	Established
BC CT1: Security, Privacy & Digital Trust (p. 140)	Canonical	Established
BC CT1: Health Research & Innovation (p. 159)	Canonical	Established
BC CT1: Health Service Direction & Stewardship (p. 165)	Canonical	Established
BC CT1: Workforce & Organisational Capability (p. 176)	Canonical	Established
BC CT1: Corporate Resources & Enterprise Services (p. 185)	Canonical	Established
BC CT1: Health Service Governance (p. 195)	Canonical	Established
BC CT1: Health Service Assurance (p. 202)	Canonical	Established

2. BC CT1: Individual Care Delivery

Property	Value	Meaning
Reference Id	`HCM-BC-individual-care-delivery`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:148 (p. 1333); business:152 (p. 1333); business:120 (p. 1332)	Source standing does not approve the derived assertion.

2.1. Capability Overview

Individual Care Delivery is the organisation's ability to understand an individual's health and act on that understanding through purposeful care. Its six constituent abilities distinguish establishing findings, interpreting their clinical meaning, choosing a response, performing care, observing change and evaluating care against its objectives. These distinctions describe responsibilities and outcomes rather than a compulsory sequence: a consultation or continuing episode may revisit several of them.

The same ability can be exercised in a hospital, a general practice, a home or a virtual consultation. Setting and time pattern affect the circumstances of care without changing the responsibility represented by this capability.

2.2. Definition / governed semantic detail

Assess, clinically determine, plan, intervene, monitor and review individual care independently of setting, mode, time pattern or organisational arrangement.

2.3. Intent

Support clinically meaningful care whose intent, performance, observed effects and continuing appropriateness can be distinguished.

2.4. Boundary — v0.1 explanation

Care access, appointments and operational coordination enable care but do not establish that it has been performed. Clinical judgement and the authority to deliver care remain with the

responsible participants; possession or coordination of their information does not transfer that authority to Harmonia.

2.5. Harmonia Architectural Relevance

Harmonia supplies and connects information used in individual care. The approved Health Information Access support trace is bounded: records can inform care, but direct assessment can establish findings independently, and access to a proposed intervention does not authorise its performance.

2.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Individual Health Assessment (p. 6)	capability CT2	Canonical / Established
BC CT2: Clinical Determination (p. 10)	capability CT2	Canonical / Established
BC CT2: Individual Care Planning (p. 14)	capability CT2	Canonical / Established
BC CT2: Clinical Intervention (p. 18)	capability CT2	Canonical / Established
BC CT2: Individual Health Monitoring (p. 19)	capability CT2	Canonical / Established
BC CT2: Individual Care Review (p. 22)	capability CT2	Canonical / Established

2.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established
To: enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
To: enabled by	BEC CT2: Care Plan Administration (p. 455)	Proposed

2.8. BC CT2: Individual Health Assessment

Property	Value	Meaning
Reference Id	`HCM-BC-individual-health-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:160 (p. 1333); business:162 (p. 1333); business:316 (p. 1335); business:320 (p. 1335); business:330 (p. 1335)	Source standing does not approve the derived assertion.

2.8.1. Capability Overview

Assessment establishes what is currently known about an individual's health through history, examination and measurement. Within Individual Care Delivery it supplies assessed findings, including findings obtained directly from the person rather than retrieved from an existing record. Its three children distinguish ways of establishing knowledge, rather than professions, clinical specialties or locations.

2.8.2. Definition / governed semantic detail

The capability to establish an understanding of an individual's current health state through the collection, consideration and evaluation of relevant clinical, health and contextual information.

2.8.3. Intent

Give subsequent clinical interpretation a defensible account of the person's current state and relevant history.

2.8.4. Boundary

Finding an abnormal measurement is distinguishable from deciding what it means clinically. Diagnosis, prognosis, selection of care and performance of care belong to adjacent abilities.

2.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health History Assessment (p. 7)	capability CT3	Canonical / Established
BC CT3: Clinical Examination (p. 8)	capability CT3	Canonical / Established
BC CT3: Health Status Measurement (p. 9)	capability CT3	Canonical / Established

2.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established

2.8.7. BC CT3: Health History Assessment

Property	Value	Meaning
Reference Id	`HCM-BC-health-history-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-HEALTH-HISTORY-ASSESSMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Health Assessment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:326 (p. 1335); business:322 (p. 1335); business:330 (p. 1335)	Source standing does not approve the derived assertion.

2.8.7.1. Capability Overview

Health History Assessment establishes clinically relevant understanding from a person's history and its surrounding context. Within assessment it contributes knowledge that a current examination or measurement alone may not reveal, while requiring consideration of the relevance of historical information to the present situation. Access to previous records can assist, but the responsibility is the assessment of history rather than retrieval itself.

2.8.7.2. Definition / governed semantic detail

Establish relevant health knowledge from the individual's history and applicable historical/contextual information.

2.8.7.3. Boundary

Historical information does not become a current clinical conclusion merely because it is available; interpretation of the assessed findings remains Clinical Determination.

2.8.8. BC CT3: Clinical Examination

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-examination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-CLINICAL-EXAMINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Health Assessment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:327 (p. 1335); business:322 (p. 1335); business:330 (p. 1335)	Source standing does not approve the derived assertion.

2.8.8.1. Capability Overview

Clinical Examination establishes findings through examination of the individual. It distinguishes directly acquired clinical knowledge from the historical account and the measurement-focused contribution of its assessment siblings. Its outcome is a set of examination findings that can inform clinical interpretation, irrespective of the setting or profession exercising the ability.

2.8.8.2. Definition / governed semantic detail

Establish health findings through clinical examination.

2.8.8.3. Boundary

An examination finding is not itself a diagnosis, care plan or performed treatment.

2.8.9. BC CT3: Health Status Measurement

Property	Value	Meaning
Reference Id	`HCM-BC-health-status-measurement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-HEALTH-STATUS-MEASUREMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Health Assessment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:328 (p. 1335); business:322 (p. 1335); business:330 (p. 1335)	Source standing does not approve the derived assertion.

2.8.9.1. Capability Overview

Health Status Measurement establishes observations or measurements of an individual's health. Its contribution to assessment is evidence about the measured state, with a purpose distinct from developing a view over time or deciding the clinical meaning of a result. Measurements can inform both present assessment and later monitoring without collapsing those abilities.

2.8.9.2. Definition / governed semantic detail

Establish individual health observations or measurements.

2.8.9.3. Boundary — v0.1 explanation

Obtaining a value does not establish a temporal trend or the clinical significance of that value.

2.9. BC CT2: Clinical Determination

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-determination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:186 (p. 1333); business:188 (p. 1333)	Source standing does not approve the derived assertion.

2.9.1. Capability Overview

Clinical Determination turns assessed findings into clinically interpreted understanding of a person's condition, risk or prognosis. It represents the professional responsibility for deciding what the findings mean in the individual's clinical context, including the significance of changes identified through monitoring. This is the interpretive responsibility within Individual Care Delivery, rather than another method of collecting evidence.

2.9.2. Definition / governed semantic detail

The capability to interpret assessed health information and establish clinically meaningful conclusions about an individual's health state, condition, risk or prognosis.

2.9.3. Intent

Establish clinical conclusions on which an intended response can be based.

2.9.4. Boundary

Determining clinical meaning does not itself select the care response. No CT3 taxonomy has been adjudicated; diagnosis and prognostic judgement illustrate this responsibility without becoming additional catalogue elements.

2.9.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Finding Interpretation (p. 11)	capability CT3	Proposed / Proposed
BC CT3: Diagnosis & Problem Determination (p. 12)	capability CT3	Proposed / Proposed
BC CT3: Health Risk & Prognostic Determination (p. 13)	capability CT3	Proposed / Proposed

2.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established

2.9.7. BC CT3: Clinical Finding Interpretation

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-finding-interpretation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION.CT3-BC-CLINICAL-FINDING-INTERPRETATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Clinical Determination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:312	Source standing does not approve the derived assertion.

2.9.7.1. Capability Overview

Interpret observed clinical findings and their significance for the individual care concern. The proposed decomposition separates this outcome from the other responsibilities within Clinical Determination.

2.9.7.2. Definition — Proposed

Interpret observed clinical findings and their significance for the individual care concern.

2.9.7.3. Intent

Make the clinical significance of observations intelligible before diagnostic or response decisions are taken.

2.9.7.4. Boundary

Does not itself establish a diagnosis, prescribe intervention or confer this clinical responsibility on Harmonia.

2.9.7.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

2.9.7.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:312

2.9.7.7. Prior Candidate Derivation

v0.3 reconciliation: `BC-20CA91E6` (line 139). Working locators are not identity aliases.

2.9.8. BC CT3: Diagnosis & Problem Determination

Property	Value	Meaning
Reference Id	`HCM-BC-diagnosis-problem-determination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION.CT3-BC-DIAGNOSIS-PROBLEM-DETERMINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Clinical Determination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:312	Source standing does not approve the derived assertion.

2.9.8.1. Capability Overview

Establish and qualify the clinical problems or diagnoses applicable to an individual. The proposed decomposition separates this outcome from the other responsibilities within Clinical Determination.

2.9.8.2. Definition — Proposed

Establish and qualify the clinical problems or diagnoses applicable to an individual.

2.9.8.3. Intent

Establish the clinically accountable problem statement that subsequent care decisions address.

2.9.8.4. Boundary

Clinical determination remains with the mandated professional; a coded record is not evidence that Harmonia made the diagnosis.

2.9.8.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

2.9.8.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:312

2.9.8.7. Prior Candidate Derivation

v0.3 reconciliation: `BC-A1AC4A2A` (line 150). Working locators are not identity aliases.

2.9.9. BC CT3: Health Risk & Prognostic Determination

Property	Value	Meaning
Reference Id	`HCM-BC-health-risk-prognostic-determination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION.CT3-BC-HEALTH-RISK-PROGNOSTIC-DETERMINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Clinical Determination	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:312	Source standing does not approve the derived assertion.

2.9.9.1. Capability Overview

Establish clinically meaningful risk and expected-course judgements for an individual. The proposed decomposition separates this outcome from the other responsibilities within Clinical Determination.

2.9.9.2. Definition — Proposed

Establish clinically meaningful risk and expected-course judgements for an individual.

2.9.9.3. Intent

Make anticipated clinical risk and course explicit for the individual care concern.

2.9.9.4. Boundary

Risk/prognostic judgement is distinct from operational urgency, source-information authority and platform assurance.

2.9.9.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

2.9.9.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:312

2.9.9.7. Prior Candidate Derivation

v0.3 reconciliation: `BC-75E01B0D` (line 161). Working locators are not identity aliases.

2.10. BC CT2: Individual Care Planning

Property	Value	Meaning
Reference Id	`HCM-BC-individual-care-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:211 (p. 1334); business:213 (p. 1334)	Source standing does not approve the derived assertion.

2.10.1. Capability Overview

Individual Care Planning establishes the intended course of care in response to assessed and clinically interpreted health information. It connects clinical understanding to objectives, intended interventions and monitoring needs. Its outcome is an expression of what should happen, allowing intended care to remain distinguishable from requests, operational arrangements and actions subsequently performed.

2.10.2. Definition / governed semantic detail

The capability to establish the intended clinical response to an individual's assessed and clinically interpreted health state, including the objectives, interventions and monitoring required to progress their care.

2.10.3. Intent

Make the clinical response and its objectives explicit enough to guide care and later evaluate its effectiveness.

2.10.4. Boundary

Prescribing can express clinical intent without establishing dispensing or administration. Scheduling or allocating resources does not perform the planned intervention. No CT3 decomposition is established.

2.10.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Care Goal Definition (p. 16)	capability CT3	Proposed / Proposed
BC CT3: Care Response Planning (p. 17)	capability CT3	Proposed / Proposed

2.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established

2.10.7. BC CT3: Care Goal Definition

Property	Value	Meaning
Reference Id	`HCM-BC-care-goal-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING.CT3-BC-CARE-GOAL-DEFINITION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Care Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:312	Source standing does not approve the derived assertion.

2.10.7.1. Capability Overview

Establish the intended outcomes and priorities against which an individual care response is planned. The proposed decomposition separates this outcome from the other responsibilities within Individual Care Planning.

2.10.7.2. Definition — Proposed

Establish the intended outcomes and priorities against which an individual care response is planned.

2.10.7.3. Intent

Establish the intended clinical benefit and priorities against which care responses can be considered.

2.10.7.4. Boundary

Goal definition remains distinct from selecting an intervention and from recording an externally agreed care plan.

2.10.7.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

2.10.7.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:312

2.10.7.7. Prior Candidate Derivation

v0.3 reconciliation: `BC-E8BBA49D` (line 183). Working locators are not identity aliases.

2.10.8. BC CT3: Care Response Planning

Property	Value	Meaning
Reference Id	`HCM-BC-care-response-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING.CT3-BC-CARE-RESPONSE-PLANNING`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Care Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:312	Source standing does not approve the derived assertion.

2.10.8.1. Capability Overview

Determine the intended clinical response and its sequencing in relation to agreed care goals. The proposed decomposition separates this outcome from the other responsibilities within Individual Care Planning.

2.10.8.2. Definition — Proposed

Determine the intended clinical response and its sequencing in relation to agreed care goals.

2.10.8.3. Intent

Make the intended clinical response coherent in relation to the agreed care goals.

2.10.8.4. Boundary

Planning is distinct from intervention delivery and from the operational definition of a synthetic activity.

2.10.8.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

2.10.8.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:312

2.10.8.7. Prior Candidate Derivation

v0.3 reconciliation: `BC-3626AD34` (line 194). Working locators are not identity aliases.

2.11. BC CT2: Clinical Intervention

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-intervention`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-INTERVENTION`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:237 (p. 1334); business:239 (p. 1334)	Source standing does not approve the derived assertion.

2.11.1. Capability Overview

Clinical Intervention is the ability to perform a clinical action under an established care intent. It marks the transition from intended care to care actually performed, whether the action seeks to alter, maintain or otherwise manage health. Its exercise can establish that treatment occurred even when the desired clinical result was not achieved.

2.11.2. Definition / governed semantic detail

The capability to perform clinical actions intended to alter, maintain or otherwise manage an individual's health state in accordance with an established care intent.

2.11.3. Intent

Put the intended clinical response into practice through appropriately authorised care.

2.11.4. Boundary

Orders and appointments express or arrange future activity; they are not evidence that the intervention occurred. Therapy, medication and procedural examples do not constitute an approved three-child taxonomy: that proposed decomposition remains provisional.

2.11.5. Preserved v0.1 Review Matters

DN-BC-010 retains the intervention candidate as provisional pending a coherent peer taxonomy.

2.11.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established

2.12. BC CT2: Individual Health Monitoring

Property	Value	Meaning
Reference Id	`HCM-BC-individual-health-monitoring`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:263 (p. 1334); business:265 (p. 1334); business:332 (p. 1335); business:336 (p. 1336); business:345 (p. 1336)	Source standing does not approve the derived assertion.

2.12.1. Capability Overview

Monitoring develops a view of an individual's health over time so that change, stability or deviation can be recognised. Health State Monitoring establishes the aggregate temporally situated view; Care Response Monitoring considers that view in relation to a particular condition and treatment. Together they distinguish observation over time from a single measurement and from the clinical judgement prompted by a trend.

2.12.2. Definition / governed semantic detail

The capability to observe and evaluate an individual's health state over time in order to identify clinically meaningful change, stability or deviation from an expected state or trajectory.

2.12.3. Intent

Make relevant trajectories visible while preserving what was observed and what still requires interpretation.

2.12.4. Boundary — v0.1 explanation

Monitoring does not establish the clinical significance of a change or judge whether care achieved its objectives. Those responsibilities remain Clinical Determination and Individual Care Review respectively.

2.12.5. Preserved v0.1 Review Matters

Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

2.12.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health State Monitoring (p. 20)	capability CT3	Canonical / Established
BC CT3: Care Response Monitoring (p. 21)	capability CT3	Canonical / Established

2.12.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established

2.12.8. BC CT3: Health State Monitoring

Property	Value	Meaning
Reference Id	`HCM-BC-health-state-monitoring`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-HEALTH-STATE-MONITORING`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Health Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:342 (p. 1336); business:338 (p. 1336); business:345 (p. 1336)	Source standing does not approve the derived assertion.

2.12.8.1. Capability Overview

Health State Monitoring brings applicable monitored data into an aggregate view situated in time. Its purpose is to make the individual's changing or stable health state intelligible across observations, rather than leaving each observation isolated. Within monitoring it provides the broader view that can also be considered in relation to a particular treatment or condition.

2.12.8.2. Definition / governed semantic detail

Establish an aggregate, temporally situated view of the individual's health from applicable monitored data.

2.12.8.3. Boundary

Aggregation is not clinical interpretation. Care Response Monitoring uses the established view rather than independently recreating it.

2.12.9. BC CT3: Care Response Monitoring

Property	Value	Meaning
Reference Id	`HCM-BC-care-response-monitoring`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-CARE-RESPONSE-MONITORING`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Health Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:343 (p. 1336); business:338 (p. 1336); business:345 (p. 1336)	Source standing does not approve the derived assertion.

2.12.9.1. Capability Overview

Care Response Monitoring considers the monitored health-state view in relation to a particular problem or condition and the treatment applied. It makes a response trajectory visible in its treatment context, helping distinguish a general change in health from a change relevant to the care being followed. It consumes the aggregate view established through Health State Monitoring.

2.12.9.2. Definition / governed semantic detail

Scope that health-state view to a particular problem/condition and the treatment applied to it.

2.12.9.3. Boundary

Identifying the response does not judge treatment effectiveness against objectives or determine a replacement treatment.

2.13. BC CT2: Individual Care Review

Property	Value	Meaning
Reference Id	`HCM-BC-individual-care-review`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:289 (p. 1335); business:291 (p. 1335); business:347 (p. 1336); business:351 (p. 1336); business:360 (p. 1336)	Source standing does not approve the derived assertion.

2.13.1. Capability Overview

Individual Care Review asks whether care is achieving its intended objectives and whether the existing care intent remains suitable. Its children separate effectiveness from appropriateness: an intervention may have an effect yet no longer be suitable to continue, or remain a reasonable choice despite an inadequate response. Review therefore evaluates the relationship between care, outcomes and intent rather than simply repeating assessment.

2.13.2. Definition / governed semantic detail

The capability to evaluate the progress and effectiveness of an individual's care against its intended objectives and establish the continuing appropriateness of the current care intent.

2.13.3. Intent

Establish whether continuing the current approach is justified and whether reconsideration is needed.

2.13.4. Boundary — v0.1 explanation

Recognising a need to change care does not establish the replacement plan. Monitoring supplies relevant observations; planning establishes revised intent.

2.13.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Care Appropriateness Review (p. 23)	capability CT3	Canonical / Established
BC CT3: Care Effectiveness Review (p. 24)	capability CT3	Canonical / Established

2.13.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Access (p. 701)	Established

2.13.7. BC CT3: Care Appropriateness Review

Property	Value	Meaning
Reference Id	`HCM-BC-care-appropriateness-review`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-APPROPRIATENESS-REVIEW`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Care Review	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:357 (p. 1336); business:353 (p. 1336); business:360 (p. 1336)	Source standing does not approve the derived assertion.

2.13.7.1. Capability Overview

Care Appropriateness Review evaluates whether the current care intent remains suitable for the individual in their present context. Its question concerns continuing suitability rather than whether treatment has produced a measurable effect. That distinction permits a review to recognise that effective care may no longer be appropriate to continue.

2.13.7.2. Definition / governed semantic detail

Evaluate whether the current intended care remains suitable in the individual's current context.

2.13.7.3. Boundary

An appropriateness judgement can identify a need to reconsider care; establishing revised intent remains Individual Care Planning.

2.13.8. BC CT3: Care Effectiveness Review

Property	Value	Meaning
Reference Id	`HCM-BC-care-effectiveness-review`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-EFFECTIVENESS-REVIEW`	Current proposed context where marked Proposed.
Current context	BC CT1: Individual Care Delivery → BC CT2: Individual Care Review	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:358 (p. 1336); business:353 (p. 1336); business:360 (p. 1336)	Source standing does not approve the derived assertion.

2.13.8.1. Capability Overview

Care Effectiveness Review evaluates delivered care against its intended goals and outcomes. It connects observed response to the reason the care was undertaken, making a judgement that monitoring alone does not supply. An effectiveness finding can inform review of appropriateness while preserving the possibility that a reasonable care choice has not achieved its objective.

2.13.8.2. Definition / governed semantic detail

Evaluate whether delivered care is achieving or has achieved its intended goals and outcomes.

2.13.8.3. Boundary

Observed improvement is not automatically proof that the intended objectives have been achieved or that continuing care is appropriate.

3. BC CT1: Care Access & Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-care-access-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:362 (p. 1336); business:366 (p. 1336); business:120 (p. 1332)	Source standing does not approve the derived assertion.

3.1. Capability Overview

This capability makes care reachable and operationally coherent across participating services. It encompasses entering care, obtaining scheduled opportunities and usable capacity, coordinating participation and responsibility transitions, and arranging bounded support logistics. Its decomposition separates access decisions, resource use, continuity and supporting work so that movement or message delivery cannot stand in for acceptance of care responsibility.

3.2. Definition / governed semantic detail

Managing patient intake, referral triage, appointment scheduling, multidisciplinary care team coordination, cross-facility patient transfers, and care continuity across organisational boundaries.

3.3. Intent

Help a person’s care requirements progress across organisational and operational boundaries with the relevant access and continuity context intact.

3.4. Boundary

Coordination does not determine clinical work or confer clinical authority. Corporate resource governance and general hospital logistics are outside the bounded care-support responsibility.

3.5. Harmonia Architectural Relevance

Referrals, encounters, appointments, operational state and communication provide material information for continuity. Harmonia's integration and coordination support does not make it the owner of clinical handover or the originating workforce and resource facts.

3.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Care Access Management (p. 27)	capability CT2	Canonical / Established
BC CT2: Care Scheduling & Capacity Management (p. 31)	capability CT2	Canonical / Established
BC CT2: Care Coordination & Transition (p. 37)	capability CT2	Canonical / Established
BC CT2: Care Support Logistics Management (p. 41)	capability CT2	Canonical / Established
BC CT2: Service Delivery Preparedness Management (p. 44)	capability CT2	Proposed / Proposed

3.7. BC CT2: Care Access Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-access-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:370 (p. 1336); business:372 (p. 1336); business:374 (p. 1336); business:384 (p. 1337)	Source standing does not approve the derived assertion.

3.7.1. Capability Overview

Care Access Management establishes the circumstances in which an individual seeks or enters care. Intake creates the access context, referral requests another participant's involvement, and eligibility determines whether applicable access conditions are met. These are separate contributions to access: a person can be eligible without having a booking, and a received referral can remain undecided.

3.7.2. Definition / governed semantic detail

Establish and manage access to applicable care.

3.7.3. Intent

Make entry requirements and referral disposition intelligible without implying that care has already been accepted or delivered.

3.7.4. Boundary — v0.1 explanation

Eligibility does not allocate care, and referral receipt does not establish an assumption of clinical responsibility.

3.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Care Intake Management (p. 28)	capability CT3	Canonical / Established
BC CT3: Care Referral Management (p. 29)	capability CT3	Canonical / Established
BC CT3: Care Eligibility Determination (p. 30)	capability CT3	Canonical / Established

3.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Referral Administration (p. 342)	Proposed
To: enabled by	BEC CT2: Scheduling Administration (p. 355)	Proposed
To: enabled by	BEC CT2: Client-Centred Administration (p. 328)	Proposed

3.7.7. BC CT3: Care Intake Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-intake-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-INTAKE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Access Management	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:380 (p. 1336); business:376 (p. 1336); business:384 (p. 1337)	Source standing does not approve the derived assertion.

3.7.7.1. Capability Overview

Care Intake Management establishes the access context through which a person seeks or enters care. It makes the entry circumstance manageable without assuming that every intake follows a referral or already satisfies eligibility conditions. Within Care Access Management, intake is the responsibility for entry context rather than a decision that all subsequent care arrangements are complete.

3.7.7.2. Definition / governed semantic detail

Establish and manage the intake context through which an individual seeks or enters care.

3.7.7.3. Boundary — v0.1 explanation

An intake record does not establish acceptance of a referral, care responsibility or delivery of care.

3.7.8. BC CT3: Care Referral Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-referral-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-REFERRAL-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Access Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:381 (p. 1336); business:376 (p. 1336); business:384 (p. 1337)	Source standing does not approve the derived assertion.

3.7.8.1. Capability Overview

Care Referral Management concerns a request for another participant or service to assess, manage or assume applicable care responsibility. It preserves the distinction between the requester's intent and the recipient's disposition, allowing care coordination to recognise an unresolved referral rather than assume a successful handoff. Referral is therefore broader than sending a document.

3.7.8.2. Definition / governed semantic detail

Manage a request for another participant/service to assess, manage or assume applicable care responsibility.

3.7.8.3. Boundary — v0.1 explanation

Submission or receipt does not establish acceptance, an appointment, delivered care or responsibility actually assumed.

3.7.9. BC CT3: Care Eligibility Determination

Property	Value	Meaning
Reference Id	`HCM-BC-care-eligibility-determination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-ELIGIBILITY-DETERMINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Access Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:382 (p. 1336); business:376 (p. 1336); business:384 (p. 1337)	Source standing does not approve the derived assertion.

3.7.9.1. Capability Overview

Care Eligibility Determination establishes whether the conditions governing access to applicable care are met. It contributes a qualified access decision alongside intake and referral, so that an identifiable request can be distinguished from one that satisfies the relevant criteria. Its purpose is eligibility in the applicable context rather than clinical diagnosis or service allocation.

3.7.9.2. Definition / governed semantic detail

Determine whether applicable eligibility conditions for care access are satisfied.

3.7.9.3. Boundary

Eligibility alone does not reserve capacity, allocate a care place or deliver care.

3.8. BC CT2: Care Scheduling & Capacity Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-scheduling-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:386 (p. 1337); business:388 (p. 1337); business:390 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.1. Capability Overview

This capability connects care demand with scheduled opportunities, suitable care places and legitimately usable operational resources. Its children distinguish a booking, a place allocation, the readiness of that place, resource deployment, flow coordination and service-level usable capacity. An empty bed or an available practitioner is relevant evidence, but neither alone establishes that the service can accommodate care.

3.8.2. Definition / governed semantic detail

Manage scheduled opportunities, care places, usable operational capacity and care-demand resourcing.

3.8.3. Intent

Support care requirements with an operationally credible combination of time, place, resources and capacity.

3.8.4. Boundary — v0.1 explanation

Deployment consumes roster, qualification and privilege context without changing their authoritative records. Operational capacity differs from strategic capacity planning and from the unresolved collective service-readiness concern.

3.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Care Appointment Management (p. 32)	capability CT3	Canonical / Established
BC CT3: Care-Place Allocation (p. 33)	capability CT3	Canonical / Established
BC CT3: Care-Place Readiness Management (p. 34)	capability CT3	Canonical / Established
BC CT3: Care Resource Deployment (p. 34)	capability CT3	Canonical / Established
BC CT3: Operational Care Flow Coordination (p. 35)	capability CT3	Canonical / Established
BC CT3: Operational Care Capacity Management (p. 36)	capability CT3	Canonical / Established

3.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Bed & Care-Place Management (p. 569)	Proposed
To: enabled by	BEC CT2: Service Capacity Management (p. 581)	Proposed

3.8.7. BC CT3: Care Appointment Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-appointment-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-APPOINTMENT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Scheduling & Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:396 (p. 1337); business:392 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.7.1. Capability Overview

Care Appointment Management establishes a scheduled opportunity for care and maintains that opportunity as arrangements change. Within scheduling and capacity it supplies the temporal commitment around which participants can organise, while leaving the actual availability of places and resources distinguishable. A scheduled opportunity is useful coordination information even before attendance or care occurs.

3.8.7.2. Definition / governed semantic detail

Establish and manage a scheduled opportunity for care.

3.8.7.3. Boundary — v0.1 explanation

A booking does not establish attendance, intervention performance or satisfaction of diagnostic-service-specific scheduling constraints.

3.8.8. BC CT3: Care-Place Allocation

Property	Value	Meaning
Reference Id	`HCM-BC-care-place-allocation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-ALLOCATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Scheduling & Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:397 (p. 1337); business:392 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.8.1. Capability Overview

Care-Place Allocation associates a care requirement with an applicable care place. Its outcome is the allocation itself, distinct from defining the place or establishing its readiness. This separation supports reasoning about reserved, allocated and usable places without assuming that an apparently empty location is fit for the requirement.

3.8.8.2. Definition / governed semantic detail

Allocate an applicable care place to a care requirement.

3.8.8.3. Boundary — v0.1 explanation

Allocation does not establish clinical suitability beyond the applicable context or make an unready place ready.

3.8.9. BC CT3: Care-Place Readiness Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-place-readiness-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-READINESS-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Scheduling & Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:398 (p. 1337); business:392 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.9.1. Capability Overview

Care-Place Readiness Management establishes whether a particular place can be allocated or used in its operational context. It connects the place's condition with its intended use so that vacancy can be distinguished from readiness. This is a place-specific responsibility within scheduling and capacity, supporting credible allocation rather than merely counting physical spaces.

3.8.9.2. Definition / governed semantic detail

Establish and manage whether a particular care place is ready for allocation or use.

3.8.9.3. Boundary — v0.1 explanation

Readiness of one place does not establish collective readiness or usable capacity of the whole service.

3.8.10. BC CT3: Care Resource Deployment

Property	Value	Meaning
Reference Id	`HCM-BC-care-resource-deployment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-RESOURCE-DEPLOYMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Scheduling & Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:399 (p. 1337); business:392 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.10.1. Capability Overview

Care Resource Deployment applies legitimately available resources to a current or planned care requirement. It considers coverage, role, qualification, engagement and privilege context to distinguish availability in principle from deployment to a particular need. Its contribution is the operational application of resources, without changing the authoritative facts that make their use legitimate.

3.8.10.2. Definition / governed semantic detail

Apply legitimately available and appropriately authorised resources to a current or planned operational care requirement, particularly using roster/on-call and service-coverage context.

3.8.10.3. Boundary

Deployment is neither roster authoring nor clinical work assignment and does not confer clinical authority or ownership of enterprise assets.

3.8.11. BC CT3: Operational Care Flow Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-operational-care-flow-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-OPERATIONAL-CARE-FLOW-COORDINATION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Scheduling & Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:400 (p. 1337); business:392 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.11.1. Capability Overview

Operational Care Flow Coordination addresses progression of care flow within the delivery environment. It provides the coordinating responsibility around operational movement and impediments, rather than treating each scheduled opportunity or place allocation as an isolated fact. Its outcome concerns coherent flow under current conditions, not the clinical determination of what care should occur.

3.8.11.2. Definition / governed semantic detail

Coordinate operational care flow and its progression within the care-delivery environment.

3.8.11.3. Boundary

Coordinating flow does not transfer clinical work, workforce governance or general hospital-logistics authority.

3.8.12. BC CT3: Operational Care Capacity Management

Property	Value	Meaning
Reference Id	`HCM-BC-operational-care-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-OPERATIONAL-CARE-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Scheduling & Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:401 (p. 1337); business:392 (p. 1337); business:403 (p. 1337)	Source standing does not approve the derived assertion.

3.8.12.1. Capability Overview

Operational Care Capacity Management establishes and manages the service's usable ability to accommodate care under current conditions. It considers capacity as a service-level concern rather than a simple count of staff, devices or vacant beds. This supports decisions grounded in what can actually be accommodated, while keeping current capacity distinct from plans for future provision.

3.8.12.2. Definition / governed semantic detail

Understand and manage the service's usable capacity to accommodate care in its current operational context.

3.8.12.3. Boundary — v0.1 explanation

Resource availability, strategic capacity requirements and collective service readiness are distinct; the latter's placement remains unresolved.

3.9. BC CT2: Care Coordination & Transition

Property	Value	Meaning
Reference Id	`HCM-BC-care-coordination-transition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:405 (p. 1337); business:407 (p. 1337); business:409 (p. 1337); business:419 (p. 1338)	Source standing does not approve the derived assertion.

3.9.1. Capability Overview

Care Coordination & Transition supports continuity by keeping participation, actual responsibility assumption and discharge progression distinct. Care teams may collaborate while responsibility is shared or changes, but physical transit and information exchange do not themselves establish who has accepted care. The capability therefore addresses continuity of accountable participation as well as the practical circumstances of departure.

3.9.2. Definition / governed semantic detail

Coordinate care participation, actual responsibility transition and discharge.

3.9.3. Intent

Avoid gaps in care continuity caused by treating a movement or communication as an accepted responsibility transition.

3.9.4. Boundary

Harmonia may assemble handover information without managing clinical handover. Arrival, discharge readiness and physical departure are not equivalent to responsibility acceptance.

3.9.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Care-Team Coordination (p. 38)	capability CT3	Canonical / Established
BC CT3: Care Responsibility Transition (p. 39)	capability CT3	Canonical / Established
BC CT3: Care Discharge Coordination (p. 40)	capability CT3	Canonical / Established

3.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Discharge Management (p. 645)	Proposed
To: enabled by	BEC CT2: Clinical Collaboration (p. 756)	Proposed
To: enabled by	BEC CT2: Care Plan Administration (p. 455)	Proposed

3.9.7. BC CT3: Care-Team Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-care-team-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-TEAM-COORDINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Coordination & Transition	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:415 (p. 1337); business:411 (p. 1337); business:419 (p. 1338)	Source standing does not approve the derived assertion.

3.9.7.1. Capability Overview

Care-Team Coordination maintains the participating team context and collaboration around care. It supports continuity by making participation intelligible across people and organisational boundaries, without assuming that membership itself assigns every clinical responsibility. Within Care Coordination & Transition it concerns collaboration around care rather than proof of a responsibility handoff.

3.9.7.2. Definition / governed semantic detail

Coordinate participating care-team context and collaboration around care.

3.9.7.3. Boundary

Harmonia's support for team information and collaboration does not confer management of clinical handover or clinical worklists.

3.9.8. BC CT3: Care Responsibility Transition

Property	Value	Meaning
Reference Id	`HCM-BC-care-responsibility-transition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-RESPONSIBILITY-TRANSITION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Coordination & Transition	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:416 (p. 1337); business:411 (p. 1337); business:419 (p. 1338)	Source standing does not approve the derived assertion.

3.9.8.1. Capability Overview

Care Responsibility Transition establishes who has actually assumed applicable care responsibility and coordinates that transition. Its purpose is to prevent physical movement or exchanged information from being mistaken for accountable acceptance. It gives care continuity a responsibility meaning that a transport checkpoint or delivered referral cannot establish alone.

3.9.8.2. Definition / governed semantic detail

Establish and coordinate who has actually assumed applicable care responsibility.

3.9.8.3. Boundary

Patient arrival, message receipt and departure do not by themselves demonstrate accepted responsibility.

3.9.9. BC CT3: Care Discharge Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-care-discharge-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-DISCHARGE-COORDINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Coordination & Transition	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:417 (p. 1337); business:411 (p. 1337); business:419 (p. 1338)	Source standing does not approve the derived assertion.

3.9.9.1. Capability Overview

Care Discharge Coordination brings applicable discharge requirements and progression into a coherent operational context. It supports the transition out of a care circumstance while keeping readiness, physical exit and continuing responsibility distinguishable. The capability concerns coordinating those requirements rather than deciding that every clinical condition for discharge has been met.

3.9.9.2. Definition / governed semantic detail

Coordinate applicable discharge requirements and progression.

3.9.9.3. Boundary

Discharge readiness and departure are not themselves evidence that another participant has accepted care responsibility.

3.10. BC CT2: Care Support Logistics Management

Property	Value	Meaning
Reference Id	`HCM-BC-care-support-logistics-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:421 (p. 1338); business:423 (p. 1338); business:425 (p. 1338); business:435 (p. 1338)	Source standing does not approve the derived assertion.

3.10.1. Capability Overview

Care Support Logistics Management coordinates the bounded physical and operational support needed to fulfil care. Patient transit, clinical-material movement and authorised support work have different subjects and completion evidence, even when the same operational environment supports them. This capability provides their common care-support frame without absorbing every logistical activity of a healthcare enterprise.

3.10.2. Definition / governed semantic detail

Coordinate bounded operational support needed for care access and fulfilment.

3.10.3. Intent

Ensure necessary supporting activity can progress with the relevant patient, material and operational constraints visible.

3.10.4. Boundary — v0.1 explanation

Assignment does not establish a worker’s qualification or clinical authority. Enterprise supply chains, facilities and workforce governance retain their own responsibilities.

3.10.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Patient Movement Coordination (p. 42)	capability CT3	Canonical / Established
BC CT3: Clinical Material Logistics Coordination (p. 43)	capability CT3	Canonical / Established
BC CT3: Care Support Work Coordination (p. 44)	capability CT3	Canonical / Established

3.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Patient Transport (p. 632)	Proposed
To: enabled by	BEC CT2: Clinical Logistics Coordination (p. 639)	Proposed
To: enabled by	BEC CT2: Work Allocation & Dispatch (p. 607)	Proposed

3.10.7. BC CT3: Patient Movement Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-patient-movement-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-PATIENT-MOVEMENT-COORDINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Support Logistics Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:431 (p. 1338); business:427 (p. 1338); business:435 (p. 1338)	Source standing does not approve the derived assertion.

3.10.7.1. Capability Overview

Patient Movement Coordination concerns the person's physical transit and the mobility constraints relevant to it. Within care-support logistics it supports movement appropriate to the patient's circumstances without treating transport completion as a clinical transition. Its responsibility is useful wherever care requires a change of physical location.

3.10.7.2. Definition / governed semantic detail

Coordinate physical patient transit and applicable mobility constraints.

3.10.7.3. Boundary — v0.1 explanation

Transport progression does not establish clinical handover, destination acceptance of care or authority over the clinical plan.

3.10.8. BC CT3: Clinical Material Logistics Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-material-logistics-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CLINICAL-MATERIAL-LOGISTICS-COORDINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Support Logistics Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:432 (p. 1338); business:427 (p. 1338); business:435 (p. 1338)	Source standing does not approve the derived assertion.

3.10.8.1. Capability Overview

This capability coordinates clinically relevant material movement and the custody, availability or disposition needed for care or diagnostic fulfilment. It makes the material's operational journey intelligible while retaining why the material matters to the clinical activity. The responsibility differs from moving a patient and from managing the diagnostic meaning of information derived from a specimen.

3.10.8.2. Definition / governed semantic detail

Coordinate clinically relevant material movement, custody, availability or disposition needed for care or diagnostic fulfilment.

3.10.8.3. Boundary — v0.1 explanation

Bounded clinical-material logistics does not establish a general procurement, supply-chain or enterprise inventory architecture.

3.10.9. BC CT3: Care Support Work Coordination

Property	Value	Meaning
Reference Id	`HCM-BC-care-support-work-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CARE-SUPPORT-WORK-COORDINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Care Support Logistics Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:433 (p. 1338); business:427 (p. 1338); business:435 (p. 1338)	Source standing does not approve the derived assertion.

3.10.9.1. Capability Overview

Care Support Work Coordination organises authorised operational support work around care: what needs doing, who will perform it and how progress, completion or escalation is coordinated. It gives supporting activity an explicit progression responsibility, rather than assuming that issuing a request means the work occurred. The work remains bounded by applicable authority and resource constraints.

3.10.9.2. Definition / governed semantic detail

Coordinate authorised operational support work: what needs doing, who performs it and how progression, completion or escalation is coordinated.

3.10.9.3. Boundary — v0.1 explanation

Dispatch does not establish worker availability, qualification or privilege, and support coordination does not determine clinical work.

3.11. BC CT2: Service Delivery Preparedness Management

Property	Value	Meaning
Reference Id	`HCM-BC-service-delivery-preparedness-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-BC-SERVICE-DELIVERY-PREPAREDNESS-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:403	Source standing does not approve the derived assertion.

3.11.1. Capability Overview

Preparedness asks whether the service as a whole can operate, distinct from scheduling capacity or readiness of a single place. This is a proposed missing business frame for the already identified readiness concern.

3.11.2. Definition — Proposed

Establish whether service offerings can be delivered in their intended operating contexts through the combined standing of their dependencies.

3.11.3. Intent

Recognise whether the whole service can actually operate in its intended context.

3.11.4. Boundary

Does not own the dependencies, their management or clinical execution.

3.11.5. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

3.11.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:403

3.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Service Operational Readiness (p. 46)	capability CT3	Candidate / Proposed

3.11.8. BC CT3: Health Service Operational Readiness

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-operational-readiness`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Candidate	Preserved Candidate concern; not promoted.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-BC-SERVICE-DELIVERY-PREPAREDNESS-MANAGEMENT.CT3-BC-HEALTH-SERVICE-OPERATIONAL-READINESS`	Current proposed context where marked Proposed.
Current context	BC CT1: Care Access & Coordination → BC CT2: Service Delivery Preparedness Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1201 (p. 1358)	Source standing does not approve the derived assertion.

3.11.8.1. Capability Overview

Collective readiness is an access prerequisite and has a coherent proposed preparedness frame; it is not reduced to capacity, resource state or service alignment.

3.11.8.2. Semantic elaboration — Proposed

Establish collective operational readiness of a service offering in a site or session context from its applicable dependencies.

3.11.8.3. Boundary — v0.1 explanation

Tier, parent and CT1 ancestry remain unresolved. Site-level and session-level exercises do not establish separate capabilities, and no convenient parent is allocated here.

3.11.8.4. Boundary

The element remains Candidate. The definition/placement elaboration is Proposed and does not adjudicate the unresolved RAD question or transfer source-domain authority.

3.11.8.5. Alternative Considered / Review Note

Health Service Direction & Stewardship is plausible for standing site/service readiness; access-oriented placement is chosen to retain the deliverability question.

3.11.8.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

3.11.8.7. Preserved v0.1 Review Matters

OI-BC-002: collective service-readiness meaning is identified, but tier/CT2 parent/CT1 ancestry and placement remain unresolved.

4. BC CT1: Health Rights, Advocacy & Participation

Property	Value	Meaning
Reference Id	`HCM-BC-health-rights-advocacy-participation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-RIGHTS-ADVOCACY-PARTICIPATION`	Current proposed context where marked Proposed.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:437 (p. 1338); business:441 (p. 1338); business:120 (p. 1332)	Source standing does not approve the derived assertion.

4.1. Capability Overview

This capability represents the organisation's responsibility to protect patients' rights and enable their voice in health decisions. Consent, advance care directives, advocacy and consumer participation concern the legitimacy and personal consequences of care, rather than merely the availability of clinical information. The governed model retains this broad enterprise responsibility without an established subordinate capability set.

4.2. Definition / governed semantic detail

Protecting patient rights, facilitating informed consent, managing advance care directives, supporting patient advocacy, and enabling consumer participation in health decisions.

4.3. Intent

Support informed participation and respect for the individual's rights in decisions affecting their health.

4.4. Boundary

Recording a directive or restricting access can support rights without performing advocacy or managing consumer participation.

4.5. Harmonia Architectural Relevance

The established Adjacent classification reflects the use and supply of consent and record-access evidence. It does not assign the underlying rights, advocacy or participation activity to Harmonia.

5. BC CT1: Diagnostic, Therapeutic & Clinical Support Services

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-therapeutic-clinical-support-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:447 (p. 1338); business:451 (p. 1338); business:120 (p. 1332)	Source standing does not approve the derived assertion.

5.1. Capability Overview

Specialised services contribute evidence, professional interpretation, medication provision and therapeutic support to individual care. The established decomposition distinguishes diagnostic provision from medication and pharmacy services, while retaining wider therapeutic and allied-health scope as enterprise context. The catalogue deliberately explains the modelled branches without presenting them as an exhaustive inventory of clinical support.

5.2. Definition / governed semantic detail

The specialised clinical services that support direct patient care, including pathology and laboratory testing, diagnostic imaging (radiology), pharmacy and medication dispensing, and allied health therapies.

5.3. Intent

Provide the specialised contributions that treating participants need to assess and care for individuals.

5.4. Boundary

Producing diagnostic evidence, authorising a report, dispensing medication and administering treatment establish different outcomes and authorities.

5.5. Harmonia Architectural Relevance

Requests, specimen associations, reports and medication events require reliable correlation and exchange. Supporting those information flows does not make Harmonia the diagnostic practitioner, pharmacy or therapy provider.

5.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Diagnostic Support Provision (p. 51)	capability CT2	Canonical / Established
BC CT2: Medication & Pharmacy Services (p. 58)	capability CT2	Canonical / Established

5.7. BC CT2: Diagnostic Support Provision

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-support-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:457 (p. 1338); business:459 (p. 1338); business:461 (p. 1338); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.1. Capability Overview

Diagnostic Support Provision covers the distinct responsibilities through which a diagnostic service obtains evidence and makes its findings available. Diagnostic scheduling, service performance, specimen stewardship, data management, interpretation, report governance and report distribution each establish a different outcome. Their separation allows preliminary, amended or final information to retain its professional and evidentiary standing without treating delivery as authorisation.

5.7.2. Definition / governed semantic detail

Provide diagnostic services and manage their evidence, interpretation and governed reporting.

5.7.3. Intent

Make diagnostic evidence and professionally governed findings usable by the appropriate care participants.

5.7.4. Boundary

Diagnostic interpretation is limited to the service's professional scope; patient-level Clinical Determination considers the wider clinical context. The children do not prescribe a universal diagnostic workflow or establish a Diagnostic Order Management capability.

5.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Diagnostic Service Appointment Management (p. 52)	capability CT3	Canonical / Established
BC CT3: Diagnostic Service Delivery (p. 53)	capability CT3	Canonical / Established
BC CT3: Diagnostic Specimen Management (p. 54)	capability CT3	Canonical / Established
BC CT3: Diagnostic Data Management (p. 55)	capability CT3	Canonical / Established
BC CT3: Diagnostic Interpretation (p. 56)	capability CT3	Canonical / Established
BC CT3: Diagnostic Report Management (p. 56)	capability CT3	Canonical / Established
BC CT3: Diagnostic Report Distribution (p. 57)	capability CT3	Canonical / Established

5.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Diagnostic Administration (p. 399)	Proposed
To: enabled by	BEC CT3: Diagnostic Service Integration (p. 401)	Proposed
To: enabled by	BEC CT2: Order Administration (p. 383)	Proposed

5.7.7. BC CT3: Diagnostic Service Appointment Management

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-service-appointment-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SERVICE-APPOINTMENT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:467 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.7.1. Capability Overview

Diagnostic Service Appointment Management makes a scheduled diagnostic opportunity credible in the service's own context. Preparation, modality, urgency and duration can materially affect whether an appointment is usable, so the responsibility is richer than recording a generic time slot. It supports service access while remaining separate from performance of the diagnostic activity.

5.7.7.2. Definition / governed semantic detail

Manage the diagnostic-service-specific scheduled opportunity, including preparation, modality, urgency, duration and other service-specific constraints.

5.7.7.3. Boundary

A diagnostic booking establishes neither evidence acquisition nor a report and is distinct from generic Care Appointment Management.

5.7.8. BC CT3: Diagnostic Service Delivery

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-service-delivery`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SERVICE-DELIVERY`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:468 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.8.1. Capability Overview

Diagnostic Service Delivery performs the activity that obtains or produces diagnostic evidence. Within Diagnostic Support Provision it is the point at which the diagnostic procedure actually occurs, rather than being scheduled or requested. The evidence obtained can then be managed and interpreted under the distinct responsibilities that give it professional meaning.

5.7.8.2. Definition / governed semantic detail

Perform the diagnostic activity that obtains or produces diagnostic evidence.

5.7.8.3. Boundary

Performance does not itself establish interpretation, report authorisation or report delivery.

5.7.9. BC CT3: Diagnostic Specimen Management

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-specimen-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SPECIMEN-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:469 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.9.1. Capability Overview

Diagnostic Specimen Management preserves the specimen context needed to trust its use: which specimen it is, whether it remains suitable, and how integrity and custody are maintained. It concerns the physical diagnostic subject across its applicable lifecycle rather than only the results generated from it. This distinction protects the relationship between evidence and the material from which it arose.

5.7.9.2. Definition / governed semantic detail

Maintain a specimen’s identity, integrity, suitability, custody and applicable lifecycle context.

5.7.9.3. Boundary

Specimen stewardship is separate from diagnostic-data management and interpretation of results.

5.7.10. BC CT3: Diagnostic Data Management

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-data-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-DATA-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:470 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.10.1. Capability Overview

Diagnostic Data Management maintains acquired or produced diagnostic evidence as usable information. Its responsibility includes identity, association, integrity, organisation and lifecycle so that evidence can be found and understood in the right diagnostic context. It addresses the evidence record independently of the professional conclusion subsequently drawn from it.

5.7.10.2. Definition / governed semantic detail

Manage acquired or produced diagnostic evidence as information, including identity, association, integrity, organisation, availability and lifecycle.

5.7.10.3. Boundary

Managing diagnostic data neither interprets that data nor governs the authoritative expression of findings in a report.

5.7.11. BC CT3: Diagnostic Interpretation

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-interpretation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-INTERPRETATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:471 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.11.1. Capability Overview

Diagnostic Interpretation establishes what diagnostic evidence demonstrates within the service's professional scope. It turns evidence into diagnostic findings without claiming the wider patient-level judgement that considers all clinical circumstances. Separating interpretation from acquisition and reporting makes the professional contribution clear even when one service performs all three.

5.7.11.2. Definition / governed semantic detail

Establish what diagnostic evidence demonstrates within the diagnostic service's professional scope.

5.7.11.3. Boundary

Patient-level Clinical Determination remains distinct, as do report authorisation and distribution.

5.7.12. BC CT3: Diagnostic Report Management

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-report-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:472 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.12.1. Capability Overview

Diagnostic Report Management governs the authoritative expression of diagnostic findings. Creation, review, verification, amendment, finalisation and signing matter because they establish the standing of the report rather than merely its existence as data. The responsibility preserves a meaningful distinction between evidence, interpretation and the report that a service is prepared to authorise.

5.7.12.2. Definition / governed semantic detail

Establish and govern the authoritative expression of diagnostic findings, including applicable creation, review, amendment, verification, finalisation and authorisation/signing.

5.7.12.3. Boundary — v0.1 explanation

Signing is behaviour within this capability, not another CT3. Sending a report does not establish its authorisation.

5.7.13. BC CT3: Diagnostic Report Distribution

Property	Value	Meaning
Reference Id	`HCM-BC-diagnostic-report-distribution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-DISTRIBUTION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Diagnostic Support Provision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:473 (p. 1339); business:463 (p. 1339); business:475 (p. 1339)	Source standing does not approve the derived assertion.

5.7.13.1. Capability Overview

Diagnostic Report Distribution provides an already established report to applicable authorised recipients or consuming contexts. Its purpose is to make the governed findings available where they are needed, while respecting access, privacy, security and communication controls. Distribution retains the report's standing rather than originating or silently reinterpreting its findings.

5.7.13.2. Definition / governed semantic detail

Provide an established report to applicable authorised recipients or consuming contexts under applicable access, privacy, security and communication controls.

5.7.13.3. Boundary — v0.1 explanation

Delivery does not establish clinical review, report authorisation or a patient-level clinical decision.

5.8. BC CT2: Medication & Pharmacy Services

Property	Value	Meaning
Reference Id	`HCM-BC-medication-pharmacy-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:477 (p. 1339); business:479 (p. 1339); business:481 (p. 1339); business:492 (p. 1339); business:1184 (p. 1358)	Source standing does not approve the derived assertion.

5.8.1. Capability Overview

This capability frames professional medication provision and medication-specific advice. Prescription dispensing, non-prescription provision, advice and dose administration aids answer different needs, even where exercised by one pharmacy. The decomposition keeps the authorised regimen, provision of medicine and actual administration separate so that a supply event is not mistaken for treatment performed.

5.8.2. Definition / governed semantic detail

Provide professional medication/pharmacy services under applicable controls.

5.8.3. Intent

Support safe, professionally controlled access to and use of medication within the established pharmacy responsibility.

5.8.4. Boundary — v0.1 explanation

Advice does not confer prescribing authority, and dose aids do not establish a regimen. Whether specialised compounding/preparation warrants a separate capability remains unresolved under OI-BC-001.

5.8.5. Preserved v0.1 Review Matters

OI-BC-001: a compounding/preparation CT3 is neither established nor rejected; do not infer subsumption.

5.8.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Prescription Medication Dispensing (p. 60)	capability CT3	Canonical / Established
BC CT3: Non-Prescription Medication Provision (p. 60)	capability CT3	Canonical / Established
BC CT3: Medication Advice (p. 61)	capability CT3	Canonical / Established
BC CT3: Medication Dose Administration Aid Provision (p. 62)	capability CT3	Canonical / Established
BC CT3: Medication Compounding & Preparation (p. 63)	capability CT3	Proposed / Proposed

5.8.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Medication Administration (p. 417)	Proposed
To: enabled by	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Proposed

5.8.8. BC CT3: Prescription Medication Dispensing

Property	Value	Meaning
Reference Id	`HCM-BC-prescription-medication-dispensing`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-PRESCRIPTION-MEDICATION-DISPENSING`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Medication & Pharmacy Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:487 (p. 1339); business:483 (p. 1339); business:492 (p. 1339)	Source standing does not approve the derived assertion.

5.8.8.1. Capability Overview

Prescription Medication Dispensing is the professional preparation and provision of medication against an applicable authorised prescription or order. It fulfils a pharmacy responsibility grounded in the prescribed intent and dispensing requirements. The resulting dispensing event provides different evidence from the prescribing decision and from later administration to the individual.

5.8.8.2. Definition / governed semantic detail

Professionally prepare and provide medication according to an applicable authorised prescription/order and dispensing requirements.

5.8.8.3. Boundary — v0.1 explanation

Dispensing does not establish that medication was administered; unresolved specialised compounding must not be silently assigned here.

5.8.9. BC CT3: Non-Prescription Medication Provision

Property	Value	Meaning
Reference Id	`HCM-BC-non-prescription-medication-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-NON-PRESCRIPTION-MEDICATION-PROVISION`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Medication & Pharmacy Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:488 (p. 1339); business:483 (p. 1339); business:492 (p. 1339)	Source standing does not approve the derived assertion.

5.8.9.1. Capability Overview

Non-Prescription Medication Provision makes medication available where the applicable conditions do not require a prescription. It remains a professional, regulated and safety-constrained responsibility rather than an unrestricted supply transaction. Its distinction from prescription dispensing preserves the different authority basis under which provision occurs.

5.8.9.2. Definition / governed semantic detail

Provide medication not requiring an applicable prescription, subject to professional, regulatory and safety controls.

5.8.9.3. Boundary

Provision neither establishes diagnosis nor proves use or administration of the medication.

5.8.10. BC CT3: Medication Advice

Property	Value	Meaning
Reference Id	`HCM-BC-medication-advice`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-MEDICATION-ADVICE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Medication & Pharmacy Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:489 (p. 1339); business:483 (p. 1339); business:492 (p. 1339)	Source standing does not approve the derived assertion.

5.8.10.1. Capability Overview

Medication Advice provides professional guidance about medication use and its precautions, interactions, administration, adherence or handling. It supports understanding of how medication can be used safely without turning every advisory interaction into a prescribing decision. Within pharmacy services, the advice outcome is distinct from provision of the medicine itself.

5.8.10.2. Definition / governed semantic detail

Provide professional medication-specific advice about appropriate use, precautions, interactions, administration, adherence or safe handling.

5.8.10.3. Boundary

Advice does not confer authority to diagnose, prescribe or alter treatment; detailed scope remains subject to refinement.

5.8.11. BC CT3: Medication Dose Administration Aid Provision

Property	Value	Meaning
Reference Id	`HCM-BC-medication-dose-administration-aid-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-MEDICATION-DOSE-ADMINISTRATION-AID-PROVISION`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Medication & Pharmacy Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:490 (p. 1339); business:483 (p. 1339); business:492 (p. 1339)	Source standing does not approve the derived assertion.

5.8.11.1. Capability Overview

This capability prepares and supplies organised doses to support administration under an already established medication regimen. Its purpose is to make that regimen easier to follow in practice while preserving the difference between organising doses and choosing therapy. The business responsibility is not defined by any particular commercial packaging arrangement.

5.8.11.2. Definition / governed semantic detail

Prepare and provide organised medication doses supporting administration according to an already established regimen.

5.8.11.3. Boundary — v0.1 explanation

Preparing a dose aid does not establish the regimen or demonstrate that a dose was administered.

5.8.12. BC CT3: Medication Compounding & Preparation

Property	Value	Meaning
Reference Id	`HCM-BC-medication-compounding-preparation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-BC-MEDICATION-COMPOUNDING-PREPARATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Diagnostic, Therapeutic & Clinical Support Services → BC CT2: Medication & Pharmacy Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:492	Source standing does not approve the derived assertion.

5.8.12.1. Capability Overview

Prepare or compound a clinically required medication formulation under the applicable professional and product-governance mandate. The proposed decomposition separates this outcome from the other responsibilities within Medication & Pharmacy Services.

5.8.12.2. Definition — Proposed

Prepare or compound a clinically required medication formulation under the applicable professional and product-governance mandate.

5.8.12.3. Intent

Provide the required medication formulation when ordinary supply alone does not satisfy the authorised preparation need.

5.8.12.4. Boundary

Preparation differs from dispensing and administration; this proposal assigns no compounding activity to Harmonia.

5.8.12.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

5.8.12.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:492

6. BC CT1: Health Products & Clinical Technology

Property	Value	Meaning
Reference Id	`HCM-BC-health-products-clinical-technology`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:494 (p. 1340); business:498 (p. 1340); business:120 (p. 1332)	Source standing does not approve the derived assertion.

6.1. Capability Overview

This capability manages health products and clinical technology as business concerns in the care environment. Its established clinical-technology branch distinguishes knowledge of the device estate, functional state, deployment, user enablement, integration and lifecycle disposition. Product procurement and supply remain relevant context without a newly inferred Health Product Management decomposition.

6.2. Definition / governed semantic detail

Manage health products and clinical technology in the care environment, with distinct device inventory, functional/release, deployment, utilisation, integration and lifecycle responsibilities. Product procurement and supply context is retained without inferring further Health Product Management decomposition.

6.3. Intent

Keep the technology context needed for safe service activity and information exchange coherent and governed.

6.4. Boundary

Device-specific enablement, clinical privilege and use in care are different concerns. The seven established device children do not resolve the separate maintenance-membership discrepancy.

6.5. Harmonia Architectural Relevance

Integration provision and device-data integration materially affect exchanged information and its care context. Supply-chain, maintenance and clinical-use authority are not transferred to Harmonia.

6.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Clinical Technology Management (p. 66)	capability CT2	Canonical / Established

6.7. BC CT2: Clinical Technology Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-technology-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:502 (p. 1340); business:504 (p. 1340); business:506 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.1. Capability Overview

Clinical Technology Management treats a device as more than a data endpoint. Its responsibilities distinguish what device exists, what functionality and release it has, where and to whom it is deployed, how authorised users are supported, whether exchange is provisioned, what data is integrated and whether it should remain in service. These distinctions preserve the business meaning of the device estate beyond the subset of information used by Harmonia.

6.7.2. Definition / governed semantic detail

Manage clinical technology as a coherent business concern rather than only the device information Harmonia happens to use.

6.7.3. Intent

Maintain intelligible device capability, deployment and lifecycle context for clinical service use.

6.7.4. Boundary

Provisioning integration does not demonstrate that data has flowed. Monitoring, restoring device condition and deciding lifecycle disposition remain distinct; maintenance's tier membership requires adjudication.

6.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Device Inventory Management (p. 67)	capability CT3	Canonical / Established
BC CT3: Clinical Device Feature & Release Management (p. 68)	capability CT3	Canonical / Established
BC CT3: Clinical Device Loan & Deployment Management (p. 69)	capability CT3	Canonical / Established
BC CT3: Clinical Device Utilisation Support (p. 70)	capability CT3	Canonical / Established
BC CT3: Clinical Device Integration Provision (p. 71)	capability CT3	Canonical / Established
BC CT3: Clinical Device Data Integration (p. 71)	capability CT3	Canonical / Established
BC CT3: Clinical Device Lifecycle Management (p. 72)	capability CT3	Canonical / Established
BC CT3: Clinical Device Maintenance Management (p. 73)	capability CT3	Candidate / Proposed

6.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Clinical Device Administration (p. 305)	Proposed
To: enabled by	BEC CT3: Device Utilisation Administration (p. 311)	Proposed

6.7.7. BC CT3: Clinical Device Inventory Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-inventory-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-INVENTORY-MANAGEMENT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:512 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.7.1. Capability Overview

Clinical Device Inventory Management establishes the managed identity and inventory context of the device estate. Specification, ownership and custody make it possible to distinguish which devices exist from their current release, deployment or use. That stable asset context supports subsequent technology-management responsibilities without claiming their different outcomes.

6.7.7.2. Definition / governed semantic detail

Establish what devices exist and their managed asset identity, specification, ownership/custody and inventory context.

6.7.7.3. Boundary — v0.1 explanation

Inventory presence does not establish readiness, functional release state or current assignment to a care requirement.

6.7.8. BC CT3: Clinical Device Feature & Release Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-feature-release-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-FEATURE-RELEASE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:513 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.8.1. Capability Overview

This capability governs the functional state of a device, including applicable software, firmware, configuration and enabled characteristics. Two instances of the same model may therefore have materially different capabilities in service. The responsibility makes those differences intelligible without reducing device governance to physical inventory or location.

6.7.8.2. Definition / governed semantic detail

Manage functional/release state, including applicable software, firmware, configuration and enabled device characteristics.

6.7.8.3. Boundary

Release state is distinct from maintenance condition, deployment and professional authority to use the device.

6.7.9. BC CT3: Clinical Device Loan & Deployment Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-loan-deployment-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LOAN-DEPLOYMENT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:514 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.9.1. Capability Overview

Clinical Device Loan & Deployment Management manages where devices are placed and the issue, assignment, return or loan context under which they are available. It connects an identifiable device with its operational deployment rather than assuming ownership or inventory

presence explains its current use. This supports an intelligible account of the device estate as placements change.

6.7.9.2. Definition / governed semantic detail

Manage device loan, issue, assignment, placement, return and deployment.

6.7.9.3. Boundary

Deployment is not device-specific user enablement, clinical use or evidence of clinical privilege.

6.7.10. BC CT3: Clinical Device Utilisation Support

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-utilisation-support`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-UTILISATION-SUPPORT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:515 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.10.1. Capability Overview

Clinical Device Utilisation Support helps otherwise appropriately authorised users use a particular device safely and effectively. It represents the device-specific enablement needed beyond general professional qualifications. Within technology management it addresses the practical support for use, without making that support the clinical action performed with the device.

6.7.10.2. Definition / governed semantic detail

Enable and support otherwise appropriately authorised users to use the particular device safely and effectively.

6.7.10.3. Boundary — v0.1 explanation

Professional qualification or privilege alone does not establish device-specific enablement; enablement does not confer clinical authority.

6.7.11. BC CT3: Clinical Device Integration Provision

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-integration-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-INTEGRATION-PROVISION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:516 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.11.1. Capability Overview

Clinical Device Integration Provision establishes the device capability and configuration required to participate in applicable information exchanges. Its outcome is an integration-ready arrangement, which must remain distinguishable from data actually acquired or exchanged. This allows an architect to discuss whether connectivity has been enabled without assuming that a particular clinical feed has operated successfully.

6.7.11.2. Definition / governed semantic detail

Establish and maintain the device’s capability and configuration to participate in applicable information exchanges.

6.7.11.3. Boundary

Provisioning does not demonstrate data integration, clinical interpretation or downstream authority for the data.

6.7.12. BC CT3: Clinical Device Data Integration

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-data-integration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-DATA-INTEGRATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:517 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.12.1. Capability Overview

Clinical Device Data Integration acquires, exchanges or conveys device data using an established integration capability. It represents actual information participation rather than the prior configuration enabling it. The distinction makes it possible to preserve the device and care context of received data while retaining separate responsibility for its subsequent interpretation.

6.7.12.2. Definition / governed semantic detail

Acquire, exchange or convey device data through established integration capability.

6.7.12.3. Boundary

Data movement does not confer clinical meaning, originating authority or device-maintenance responsibility.

6.7.13. BC CT3: Clinical Device Lifecycle Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-lifecycle-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LIFECYCLE-MANAGEMENT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:518 (p. 1340); business:508 (p. 1340); business:520 (p. 1340)	Source standing does not approve the derived assertion.

6.7.13.1. Capability Overview

Clinical Device Lifecycle Management governs whether devices enter, remain in or leave the managed estate. Introduction, review, replacement, withdrawal and retirement concern the device's overall service disposition. This gives technology management a longer-lived responsibility than the device's current loan, release or data-exchange session.

6.7.13.2. Definition / governed semantic detail

Govern introduction, continued service, review, replacement, withdrawal, decommissioning and retirement within the overall device estate.

6.7.13.3. Boundary — v0.1 explanation

Lifecycle disposition differs from monitoring and restoring device condition; the maintenance-membership discrepancy remains unresolved.

6.7.14. BC CT3: Clinical Device Maintenance Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-device-maintenance-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Candidate	Preserved Candidate concern; not promoted.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-BC-CLINICAL-DEVICE-MAINTENANCE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Products & Clinical Technology → BC CT2: Clinical Technology Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1216 (p. 1358)	Source standing does not approve the derived assertion.

6.7.14.1. Capability Overview

Maintenance/restoration is materially distinct from overall lifecycle disposition and monitoring; include the Candidate concern as a proposed eighth CT3.

6.7.14.2. Semantic elaboration — Proposed

Maintain or restore acceptable device condition through testing, calibration, inspection, maintenance and repair.

6.7.14.3. Boundary — v0.1 explanation

Membership, tier and ancestry require adjudication; this description does not establish an eighth Clinical Technology Management child.

6.7.14.4. Boundary

The element remains Candidate. The definition/placement elaboration is Proposed and does not adjudicate the unresolved RAD question or transfer source-domain authority.

6.7.14.5. Alternative Considered / Review Note

The seven-child adjudicated set omits it. Membership remains Candidate rather than silently amending that approved set.

6.7.14.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

6.7.14.7. Preserved v0.1 Review Matters

DN-BC-015 defines maintenance separately but omits it from its explicit seven-child CT3 set; inclusion/tier/ancestry requires adjudication.

7. BC CT1: Clinical Quality, Safety & Improvement

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-quality-safety-improvement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:522 (p. 1340); business:526 (p. 1340); business:120 (p. 1332)	Source standing does not approve the derived assertion.

7.1. Capability Overview

This capability addresses operational clinical quality and safety, including concerns, incidents, response and improvement. Its established safety branch distinguishes risk identification, prospective mitigation and active response. Its quality branch distinguishes criteria, evidence, assessment and reporting. Together these make it possible to reason about clinical quality without equating an observation, a judgement and the action taken in response.

7.2. Definition / governed semantic detail

Operational clinical quality, safety and improvement: monitoring clinical outcomes; identifying quality and safety concerns; adverse event and incident management, reporting and response; infection control; operational quality and safety monitoring; continuous quality improvement; and coordination of operational responses to identified concerns.

7.3. Intent

Support recognition and management of clinical safety concerns and informed improvement of clinical quality.

7.4. Boundary

This is neither a generic enterprise incident container nor a replacement for macro-level Health Service Assurance. Its bounded decomposition does not exhaust clinical quality and safety practice.

7.5. Harmonia Architectural Relevance

Information about outcomes, risks, incidents and operational responses materially supports these responsibilities. Integrating that evidence does not assign clinical quality judgement or originating clinical authority to the platform.

7.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Clinical Safety Management (p. 76)	capability CT2	Canonical / Established
BC CT2: Clinical Quality Management (p. 79)	capability CT2	Canonical / Established

7.7. BC CT2: Clinical Safety Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-safety-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:530 (p. 1340); business:532 (p. 1341); business:534 (p. 1341); business:544 (p. 1341)	Source standing does not approve the derived assertion.

7.7.1. Capability Overview

Clinical Safety Management distinguishes recognising a safety risk from reducing future exposure and actively dealing with a developing or materialised concern. These responsibilities may use overlapping evidence, but their purposes differ: characterisation makes the risk intelligible, mitigation changes prospective risk, and response addresses the situation requiring action.

7.7.2. Definition / governed semantic detail

Manage the clinical-risk distinctions materially required for R1/R2.

7.7.3. Intent

Support proportionate safety action grounded in the nature of the identified clinical risk.

7.7.4. Boundary

Monitoring evidence is not automatically risk identification. Other capabilities can perform required actions without transferring their responsibilities; the three children are not a mandatory sequence or exhaustive safety taxonomy.

7.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Risk Identification (p. 77)	capability CT3	Canonical / Established
BC CT3: Clinical Risk Mitigation Management (p. 78)	capability CT3	Canonical / Established
BC CT3: Clinical Risk Response Management (p. 79)	capability CT3	Canonical / Established

7.7.6. BC CT3: Clinical Risk Identification

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-risk-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Safety Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:540 (p. 1341); business:536 (p. 1341); business:544 (p. 1341)	Source standing does not approve the derived assertion.

7.7.6.1. Capability Overview

Clinical Risk Identification recognises and characterises a safety risk from relevant evidence. It establishes the concern to which mitigation or active response may be directed, rather than assuming that every monitored deviation is already a clinical risk. Its value is the meaningful characterisation that makes proportionate safety consideration possible.

7.7.6.2. Definition / governed semantic detail

Recognise and characterise a clinical safety risk from applicable evidence.

7.7.6.3. Boundary

Evidence observation alone does not identify risk or establish a mitigation or response decision.

7.7.7. BC CT3: Clinical Risk Mitigation Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-risk-mitigation-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-MITIGATION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Safety Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:541 (p. 1341); business:536 (p. 1341); business:544 (p. 1341)	Source standing does not approve the derived assertion.

7.7.7.1. Capability Overview

Clinical Risk Mitigation Management addresses prospective measures intended to reduce an identified risk's likelihood, exposure or consequences. It gives prevention a distinct responsibility alongside risk characterisation and active response. Required measures can involve other capabilities without transferring the ownership of their actions or information to safety management.

7.7.7.2. Definition / governed semantic detail

Manage prospective/preventative measures reducing identified risk likelihood, exposure or consequence.

7.7.7.3. Boundary

Mitigation differs from containing or remediating a concern that has materialised or requires active intervention.

7.7.8. BC CT3: Clinical Risk Response Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-risk-response-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-RESPONSE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Safety Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:542 (p. 1341); business:536 (p. 1341); business:544 (p. 1341)	Source standing does not approve the derived assertion.

7.7.8.1. Capability Overview

Clinical Risk Response Management manages active intervention, containment or remediation where a safety risk is emerging, materialised or otherwise demands action. Its purpose is to address the developing situation rather than only reduce future exposure. The responsibility remains clinical-risk-specific while the actual actions may belong to other established abilities.

7.7.8.2. Definition / governed semantic detail

Manage active response, containment and remediation where risk has materialised, is emerging or requires active intervention.

7.7.8.3. Boundary

It is not a general enterprise incident-management container and does not acquire the clinical authority of responding participants.

7.8. BC CT2: Clinical Quality Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-quality-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:546 (p. 1341); business:548 (p. 1341); business:550 (p. 1341); business:561 (p. 1341)	Source standing does not approve the derived assertion.

7.8.1. Capability Overview

Clinical Quality Management supplies the criteria and evidence basis for evaluating clinical quality and communicating established findings. Its four children preserve the difference between the expected standard, the material considered, the assessment reached and the report of that assessment. This supports scrutiny of why a quality conclusion is justified rather than treating the presence of measurements as a judgement.

7.8.2. Definition / governed semantic detail

Manage the clinical-quality distinctions materially required for R1/R2.

7.8.3. Intent

Make clinical-quality conclusions understandable in relation to their criteria, evidence and assessment authority.

7.8.4. Boundary

Reporting does not originate the conclusion or perform subsequent improvement. Neither a separate monitoring provider nor generic Continuous Improvement placement is established by this decomposition.

7.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Quality Criteria Management (p. 81)	capability CT3	Canonical / Established
BC CT3: Clinical Quality Evidence Management (p. 81)	capability CT3	Canonical / Established
BC CT3: Clinical Quality Review & Assessment Management (p. 82)	capability CT3	Canonical / Established
BC CT3: Clinical Quality Reporting (p. 83)	capability CT3	Canonical / Established

7.8.6. BC CT3: Clinical Quality Criteria Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-quality-criteria-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-CRITERIA-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Quality Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:556 (p. 1341); business:552 (p. 1341); business:561 (p. 1341)	Source standing does not approve the derived assertion.

7.8.6.1. Capability Overview

Clinical Quality Criteria Management establishes the measures, indicators and expectations used to consider clinical quality. It makes the basis of a later judgement explicit so that evidence can be assessed against an identifiable expectation rather than an unstated standard. Criteria are distinct from the measurements or findings produced when they are applied.

7.8.6.2. Definition / governed semantic detail

Establish and manage measures, indicators, expectations and criteria against which clinical quality is considered.

7.8.6.3. Boundary

Managing criteria does not originate the underlying evidence or establish a quality conclusion.

7.8.7. BC CT3: Clinical Quality Evidence Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-quality-evidence-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-EVIDENCE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Quality Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:557 (p. 1341); business:552 (p. 1341); business:561 (p. 1341)	Source standing does not approve the derived assertion.

7.8.7.1. Capability Overview

Clinical Quality Evidence Management maintains the body of material applicable to quality consideration. Its purpose is to retain evidence in a form and context that can support review without treating availability as proof of quality. The originating activity and its authority remain distinguishable from the later quality purpose for which its information is used.

7.8.7.2. Definition / governed semantic detail

Manage the body of applicable evidence used for clinical-quality consideration without owning its originating activity.

7.8.7.3. Boundary

Evidentiary use transfers neither source ownership nor authority to determine the activity being reviewed.

7.8.8. BC CT3: Clinical Quality Review & Assessment Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-quality-review-assessment-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REVIEW-ASSESSMENT-MANAGEMENT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Quality Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:558 (p. 1341); business:552 (p. 1341); business:561 (p. 1341)	Source standing does not approve the derived assertion.

7.8.8.1. Capability Overview

This capability evaluates quality evidence against applicable criteria and establishes a finding, assessment or conclusion. It is the judgement-bearing responsibility within Clinical Quality Management, distinct from maintaining the material and from communicating the result. Making that distinction supports scrutiny of the criterion and evidentiary basis for a quality assessment.

7.8.8.2. Definition / governed semantic detail

Review evidence against applicable criteria and establish a clinical-quality assessment, finding or conclusion.

7.8.8.3. Boundary

An assessment is not a report of itself or the subsequent improvement action it may inform.

7.8.9. BC CT3: Clinical Quality Reporting

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-quality-reporting`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REPORTING`	Current proposed context where marked Proposed.
Current context	BC CT1: Clinical Quality, Safety & Improvement → BC CT2: Clinical Quality Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:559 (p. 1341); business:552 (p. 1341); business:561 (p. 1341)	Source standing does not approve the derived assertion.

7.8.9.1. Capability Overview

Clinical Quality Reporting communicates established quality measures, assessments and findings to authorised consumers. It makes the outcome of quality consideration usable beyond the assessment context while retaining its basis and standing. Reporting therefore represents a communication responsibility, not another opportunity to originate or silently change the reported judgement.

7.8.9.2. Definition / governed semantic detail

Communicate established clinical-quality measures, assessments, findings and conclusions to authorised consumers.

7.8.9.3. Boundary

Communicating a quality concern does not perform the operational or clinical improvement response.

8. BC CT1: Community Health & Wellbeing

Property	Value	Meaning
Reference Id	`HCM-BC-community-health-wellbeing`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:563 (p. 1341); business:567 (p. 1341); business:120 (p. 1332)	Source standing does not approve the derived assertion.

8.1. Capability Overview

Community Health & Wellbeing frames organised activity supporting health beyond a single person's episode of care. The governed decomposition focuses on preventative health: identifying needs, defining programmes and cohorts, educating, observing programme operation and reporting. Wider outreach, environmental and social-determinant concerns remain enterprise context rather than additional modelled children.

8.2. Definition / governed semantic detail

Support community health and wellbeing, including the bounded preventative-health abilities required for R1/R2. Wider environmental health, outreach and social determinants remain enterprise context without inferred additional decomposition.

8.3. Intent

Support organised prevention with a clear relationship between the health problem, intended population and programme activity.

8.4. Boundary

Programme management does not acquire the clinical or appointment responsibilities needed to deliver a programme.

8.5. Harmonia Architectural Relevance

Programme and cohort distinctions materially shape the information, notifications and care enablement required. This supports the Relevant classification without assigning wider public-health operating authority to Harmonia.

8.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Preventative Health (p. 86)	capability CT2	Canonical / Established

8.7. BC CT2: Preventative Health

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH`	Current proposed context where marked Proposed.
Current context	BC CT1: Community Health & Wellbeing	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:571 (p. 1341); business:573 (p. 1342); business:575 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.1. Capability Overview

Preventative Health organises a response to a health problem before adverse outcomes arise or worsen. The established children distinguish why attention is warranted, how an organised programme is managed, who it targets, the education provided, what programme operation shows and what is reported. This allows a screening or immunisation programme to be explained without creating a new capability for every programme type.

8.7.2. Definition / governed semantic detail

Address preventative-health needs through organised activity with sufficient R1/R2 distinctions.

8.7.3. Intent

Connect a preventative purpose with an identifiable target population and observable organised activity.

8.7.4. Boundary

Programme performance differs from population-health assessment and individual clinical monitoring. Unmodelled specialist responsibilities are not silently assigned to the existing children.

8.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Preventative Health Problem Identification (p. 87)	capability CT3	Canonical / Established
BC CT3: Preventative Health Programme Management (p. 88)	capability CT3	Canonical / Established
BC CT3: Preventative Health Education Services (p. 89)	capability CT3	Canonical / Established
BC CT3: Preventative Health Programme Monitoring (p. 90)	capability CT3	Canonical / Established
BC CT3: Preventative Health Reporting (p. 90)	capability CT3	Canonical / Established
BC CT3: Preventative Health Cohort Management (p. 91)	capability CT3	Canonical / Established

8.7.6. BC CT3: Preventative Health Problem Identification

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health-problem-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROBLEM-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Community Health & Wellbeing → BC CT2: Preventative Health	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:581 (p. 1342); business:577 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.6.1. Capability Overview

This capability establishes the preventative-health problem warranting organised attention. It gives an intended programme a reason grounded in the concern being addressed rather than assuming that a population statistic already defines the response. The identified problem can frame subsequent cohort and programme considerations without becoming an individual's clinical diagnosis.

8.7.6.2. Definition / governed semantic detail

Identify and characterise a preventative-health problem warranting organised attention.

8.7.6.3. Boundary

Population evidence informs problem identification but does not by itself establish a programme, cohort or individual care need.

8.7.7. BC CT3: Preventative Health Programme Management

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health-programme-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Community Health & Wellbeing → BC CT2: Preventative Health	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:582 (p. 1342); business:577 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.7.1. Capability Overview

Preventative Health Programme Management establishes and manages an organised response to a preventative need or objective. It keeps the programme purpose and coordinated activity intelligible while allowing appointments, interventions and other delivery abilities to retain their own responsibilities. Screening and immunisation illustrate programmes rather than creating additional capability tiers.

8.7.7.2. Definition / governed semantic detail

Establish and manage an organised response to a preventative-health need/objective without absorbing its delivery capabilities.

8.7.7.3. Boundary

Managing the programme does not acquire the clinical or operational authority of the activities delivering it.

8.7.8. BC CT3: Preventative Health Education Services

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health-education-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-EDUCATION-SERVICES`	Current proposed context where marked Proposed.
Current context	BC CT1: Community Health & Wellbeing → BC CT2: Preventative Health	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:583 (p. 1342); business:577 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.8.1. Capability Overview

Preventative Health Education Services provides education intended to influence knowledge or behaviour so that adverse health outcomes can be prevented. Its purpose distinguishes it from general organisational communication: the educational contribution is directed to a preventative-health need. It can support organised programme activity without being equivalent to programme management or clinical intervention.

8.7.8.2. Definition / governed semantic detail

Provide education intended to prevent adverse health outcomes and influence preventative knowledge or behaviour.

8.7.8.3. Boundary — v0.1 explanation

Communicating educational material does not establish individual clinical assessment, treatment or programme effectiveness.

8.7.9. BC CT3: Preventative Health Programme Monitoring

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health-programme-monitoring`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MONITORING`	Current proposed context where marked Proposed.
Current context	BC CT1: Community Health & Wellbeing → BC CT2: Preventative Health	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:584 (p. 1342); business:577 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.9.1. Capability Overview

This capability observes how a preventative programme is operating, including reach, participation, coverage and progress. It supplies an account of programme performance in the programme's own scope, rather than substituting population-health state for evidence of programme activity. That account can inform reporting or consideration of the programme without itself being an assurance conclusion.

8.7.9.2. Definition / governed semantic detail

Observe programme operation, reach, participation, coverage, progress and applicable performance.

8.7.9.3. Boundary

Programme monitoring is distinct from individual health monitoring, population assessment and independent assurance.

8.7.10. BC CT3: Preventative Health Reporting

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health-reporting`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-REPORTING`	Current proposed context where marked Proposed.
Current context	BC CT1: Community Health & Wellbeing → BC CT2: Preventative Health	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:585 (p. 1342); business:577 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.10.1. Capability Overview

Preventative Health Reporting communicates established information about preventative problems, programmes, cohorts, activity and outcomes. It brings these different subjects together for a reporting purpose while preserving their distinct meanings. A programme's reach and an observed population outcome can both be relevant without implying that one proves the other.

8.7.10.2. Definition / governed semantic detail

Report established preventative-health problems, programmes, cohorts, activity, performance and outcomes.

8.7.10.3. Boundary

Reporting does not originate the underlying problem, cohort or evaluation conclusion.

8.7.11. BC CT3: Preventative Health Cohort Management

Property	Value	Meaning
Reference Id	`HCM-BC-preventative-health-cohort-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-COHORT-MANAGEMENT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Community Health & Wellbeing → BC CT2: Preventative Health	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:586 (p. 1342); business:577 (p. 1342); business:588 (p. 1342)	Source standing does not approve the derived assertion.

8.7.11.1. Capability Overview

Preventative Health Cohort Management establishes who belongs to the population targeted for a preventative purpose. Inclusion, exclusion, stratification and entry or exit context make membership explainable rather than a list detached from its rationale. It provides a governed population context through which programme activity and reporting can be understood.

8.7.11.2. Definition / governed semantic detail

Establish and manage the target population, inclusion/exclusion criteria, membership, stratification and applicable entry/exit context.

8.7.11.3. Boundary

A population at risk is not automatically a defined cohort, and cohort membership is not an individual clinical determination.

9. BC CT1: Population Health & Health-System Planning

Property	Value	Meaning
Reference Id	`HCM-BC-population-health-health-system-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:590 (p. 1342); business:594 (p. 1342); business:120 (p. 1332)	Source standing does not approve the derived assertion.

9.1. Capability Overview

This capability connects an understanding of population health with consideration of future health-service needs. Surveillance addresses population state, significant events and notification; planning assesses service demand and required capacity and distribution. Population evidence can inform planning without predetermining its conclusions or becoming the operational allocation of resources.

9.2. Definition / governed semantic detail

Understand population health through assessment/surveillance and applicable event/notification management; assess health-service demand and plan required capacity and its distribution.

9.3. Intent

Support informed public-health awareness and a reasoned view of the services a population will require.

9.4. Boundary

Population conclusions differ from individual clinical findings, and planned capacity differs from capacity currently usable by a service.

9.5. Harmonia Architectural Relevance

Governed population information and formal notifications materially support health activity. Consuming planning context does not make the platform the health-system planner or owner of its resources.

9.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Population Health Surveillance (p. 94)	capability CT2	Canonical / Established
BC CT2: Health-System Planning (p. 99)	capability CT2	Canonical / Established

9.7. BC CT2: Population Health Surveillance

Property	Value	Meaning
Reference Id	`HCM-BC-population-health-surveillance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:598 (p. 1342); business:600 (p. 1342); business:602 (p. 1342); business:614 (p. 1343)	Source standing does not approve the derived assertion.

9.7.1. Capability Overview

Population Health Surveillance establishes patterns and change across a defined population and manages the surveillance arrangements through which they are considered. Its children separate longitudinal assessment, surveillance management, event identification, formal notification and reporting. These distinctions matter because an observed pattern can be significant without being notifiable, and identifying a notifiable event does not demonstrate that the notification obligation has been discharged.

9.7.2. Definition / governed semantic detail

Understand population-health state and manage organised surveillance, significant events and applicable notification.

9.7.3. Intent

Make population-health concerns and applicable communication obligations visible in their population and temporal context.

9.7.4. Boundary

Notification management does not define the obligation or own the operational response. Surveillance evidence can inform other abilities without establishing their findings.

9.7.5. Preserved v0.1 Review Matters

Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

9.7.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Population Health Longitudinal Assessment (p. 95)	capability CT3	Canonical / Established
BC CT3: Population Health Surveillance Management (p. 96)	capability CT3	Canonical / Established
BC CT3: Population Health Event Identification (p. 97)	capability CT3	Canonical / Established
BC CT3: Notifiable Health Event Management (p. 98)	capability CT3	Canonical / Established
BC CT3: Population Health Reporting (p. 99)	capability CT3	Canonical / Established

9.7.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Population Health Information Reporting (p. 817)	Proposed
To: enabled by	BEC CT2: Health Information Access (p. 701)	Proposed

9.7.8. BC CT3: Population Health Longitudinal Assessment

Property	Value	Meaning
Reference Id	`HCM-BC-population-health-longitudinal-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-LONGITUDINAL-ASSESSMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Population Health Surveillance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:608 (p. 1342); business:604 (p. 1342); business:614 (p. 1343)	Source standing does not approve the derived assertion.

9.7.8.1. Capability Overview

This capability establishes population-health state, patterns and change over time from relevant evidence. It interprets information at a population scope rather than treating a collection of individual observations as a population conclusion by itself. Its longitudinal purpose makes the temporal and population basis important to the meaning of the assessment.

9.7.8.2. Definition / governed semantic detail

Establish population-health state, patterns and change over time from applicable evidence.

9.7.8.3. Boundary

Population assessment is not preventative-programme performance, an individual health conclusion or a service-demand decision.

9.7.9. BC CT3: Population Health Surveillance Management

Property	Value	Meaning
Reference Id	`HCM-BC-population-health-surveillance-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-SURVEILLANCE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Population Health Surveillance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:609 (p. 1342); business:604 (p. 1342); business:614 (p. 1343)	Source standing does not approve the derived assertion.

9.7.9.1. Capability Overview

Population Health Surveillance Management establishes the purposes, populations and regimes under which surveillance operates. It concerns the organised basis for watching health concerns, rather than any particular event detected through that activity. Keeping the regime distinct from its findings makes it possible to understand why evidence is being collected and considered.

9.7.9.2. Definition / governed semantic detail

Establish and manage surveillance purposes, populations, concerns and regimes.

9.7.9.3. Boundary

Managing surveillance neither identifies every significant event nor determines the operational response to one.

9.7.10. BC CT3: Population Health Event Identification

Property	Value	Meaning
Reference Id	`HCM-BC-population-health-event-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-EVENT-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Population Health Surveillance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:610 (p. 1342); business:604 (p. 1342); business:614 (p. 1343)	Source standing does not approve the derived assertion.

9.7.10.1. Capability Overview

Population Health Event Identification recognises and characterises a significant event or pattern from relevant evidence. It establishes the population-level concern rather than merely noting that observations changed. The resulting event context can inform notification or response decisions while retaining a separate responsibility for those decisions.

9.7.10.2. Definition / governed semantic detail

Identify and characterise significant population-health events/patterns from applicable evidence.

9.7.10.3. Boundary

An identified event is not automatically notifiable, and notifiability does not demonstrate that an obligation was discharged.

9.7.11. BC CT3: Notifiable Health Event Management

Property	Value	Meaning
Reference Id	`HCM-BC-notifiable-health-event-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-NOTIFIABLE-HEALTH-EVENT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Population Health Surveillance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:611 (p. 1342); business:604 (p. 1342); business:614 (p. 1343)	Source standing does not approve the derived assertion.

9.7.11.1. Capability Overview

Notifiable Health Event Management addresses the formal communication responsibilities applicable to a health event. It manages applicability, required information, recipient and submission status, including correction or supplementation where relevant. Its purpose is to make the notification obligation and its discharge distinguishable from both event identification and ordinary reporting.

9.7.11.2. Definition / governed semantic detail

Manage formal notification applicability, required information, recipient, status, submission, correction/supplementation and applicable follow-up.

9.7.11.3. Boundary

Notification management does not define the governing obligation or own the operational response to the event.

9.7.12. BC CT3: Population Health Reporting

Property	Value	Meaning
Reference Id	`HCM-BC-population-health-reporting`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-REPORTING`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Population Health Surveillance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:612 (p. 1342); business:604 (p. 1342); business:614 (p. 1343)	Source standing does not approve the derived assertion.

9.7.12.1. Capability Overview

Population Health Reporting communicates established assessments, surveillance findings, events and trends. Its role is to make population understanding available with its scope intact, rather than replace the assessment or event-identification work underlying it. This supports use of the findings by other abilities without assuming what those abilities must conclude.

9.7.12.2. Definition / governed semantic detail

Communicate established population-health assessments, surveillance, events, trends and findings.

9.7.12.3. Boundary — v0.1 explanation

General reporting is distinct from formal notification management and does not establish a response or planning decision.

9.8. BC CT2: Health-System Planning

Property	Value	Meaning
Reference Id	`HCM-BC-health-system-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:616 (p. 1343); business:618 (p. 1343); business:620 (p. 1343); business:630 (p. 1343)	Source standing does not approve the derived assertion.

9.8.1. Capability Overview

Health-System Planning considers the capacity and service footprint required to meet assessed demand. Demand Assessment establishes the basis and scale of need, Capacity Planning determines what is required, and Distribution Planning addresses where that capacity should sit across the service network. The decomposition preserves the difference between understanding demand and deciding a response to it.

9.8.2. Definition / governed semantic detail

Assess health-service demand and plan the capacity and footprint required to meet it.

9.8.3. Intent

Support a defensible relationship between expected demand and the planned scale and distribution of health services.

9.8.4. Boundary — v0.1 explanation

Plans do not establish available resources or operational allocations. Detailed commissioning, capital, funding and workforce planning remain deliberately unmodelled.

9.8.5. Preserved v0.1 Review Matters

Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

9.8.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Service Demand Assessment (p. 101)	capability CT3	Canonical / Established
BC CT3: Health Service Capacity Planning (p. 102)	capability CT3	Canonical / Established
BC CT3: Health Service Distribution Planning (p. 103)	capability CT3	Canonical / Established

9.8.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Service Planning Data Management (p. 818)	Proposed

9.8.8. BC CT3: Health Service Demand Assessment

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-demand-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-DEMAND-ASSESSMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Health-System Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:626 (p. 1343); business:622 (p. 1343); business:630 (p. 1343)	Source standing does not approve the derived assertion.

9.8.8.1. Capability Overview

Health Service Demand Assessment establishes a view of current or expected demand within a population, geography or service context. It translates relevant evidence into a demand judgement rather than assuming population-health state directly determines service requirements. Its assumptions and scope matter because capacity planning depends on what demand has actually been assessed.

9.8.8.2. Definition / governed semantic detail

Assess current/expected health-service demand across a population, geography, service domain or health-system context.

9.8.8.3. Boundary

System-level demand is distinct from a particular person's care requirement and does not itself determine planned capacity.

9.8.9. BC CT3: Health Service Capacity Planning

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-capacity-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-CAPACITY-PLANNING`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Health-System Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:627 (p. 1343); business:622 (p. 1343); business:630 (p. 1343)	Source standing does not approve the derived assertion.

9.8.9.1. Capability Overview

Health Service Capacity Planning determines the strategic or tactical capacity required to address assessed demand. It considers what should be available, preserving a distinction between requirement and the resources currently in service. Within Health-System Planning it gives demand a capacity response without performing operational allocation.

9.8.9.2. Definition / governed semantic detail

Determine required strategic/tactical capacity to address assessed demand.

9.8.9.3. Boundary

Planned capacity is neither established resource availability nor the service's currently usable operational capacity.

9.8.10. BC CT3: Health Service Distribution Planning

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-distribution-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-DISTRIBUTION-PLANNING`	Current proposed context where marked Proposed.
Current context	BC CT1: Population Health & Health-System Planning → BC CT2: Health-System Planning	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:628 (p. 1343); business:622 (p. 1343); business:630 (p. 1343)	Source standing does not approve the derived assertion.

9.8.10.1. Capability Overview

Health Service Distribution Planning determines how planned capacity should be distributed across a geographical, organisational or service-network footprint. It addresses the placement of the planned response rather than only its aggregate scale. This allows the planning context to distinguish enough total capacity from capacity positioned where it is needed.

9.8.10.2. Definition / governed semantic detail

Determine the distribution of planned capacity across a geographical, organisational, site or service-network footprint.

9.8.10.3. Boundary — v0.1 explanation

A planned footprint does not establish recognised location records, identifiable offerings or actual resource deployment.

10. BC CT1: Health Information & Knowledge Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-knowledge-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:636 (p. 1343); business:640 (p. 1343); business:120 (p. 1332)	Source standing does not approve the derived assertion.

10.1. Capability Overview

This capability governs both health information and reusable clinical knowledge while preserving their different meanings. Health Information Management concerns content, structures, lifecycle, discovery context, grouping, correlation and retrieval. Clinical Knowledge Management concerns identifying and formulating reusable knowledge and establishing its governed standing. A source clinical fact can contribute to knowledge without becoming a general clinical conclusion merely through reuse.

10.2. Definition / governed semantic detail

Manage governed health-information content, structures, lifecycle, catalogues, compartments, correlation and retrieval, alongside reusable clinical knowledge and its governed standing.

10.3. Intent

Keep information interpretable and usable across time and context while making the standing and limits of reusable knowledge explicit.

10.4. Boundary

Managing or preserving information does not originate its clinical facts. Governing structures and binding concepts does not acquire the authority of the terminology they reference.

10.5. Harmonia Architectural Relevance

These are substantive information-management responsibilities within Harmonia's assigned boundary. The Core classification remains bounded by information authority, active/durable-state separation and clinical decision authority.

10.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Health Information Management (p. 105)	capability CT2	Canonical / Established
BC CT2: Clinical Knowledge Management (p. 112)	capability CT2	Canonical / Established

10.7. BC CT2: Health Information Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:644 (p. 1343); business:646 (p. 1343); business:648 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.1. Capability Overview

Health Information Management maintains health information as meaningful governed content rather than isolated persisted values. Its children distinguish the content itself from the structures that explain it, lifecycle and history, discovery aids, contextual groupings, relationships and retrieval. These distinctions let an architect ask whether information exists, what it means, which state applies and whether it may be obtained without conflating those questions.

10.7.2. Definition / governed semantic detail

Manage health information as governed, semantically meaningful content.

10.7.3. Intent

Preserve usable health information whose meaning, relationships and temporal standing can be understood.

10.7.4. Boundary

Discoverability is not permission, compartment membership is not an access decision, and correlation is not identity or clinical interpretation. Current state and immutable historical operational truth remain distinct.

10.7.5. Preserved v0.1 Review Matters

Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

10.7.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Information Data Management (p. 106)	capability CT3	Canonical / Established
BC CT3: Health Information Structure Management (p. 107)	capability CT3	Canonical / Established
BC CT3: Health Information Lifecycle Management (p. 108)	capability CT3	Canonical / Established
BC CT3: Health Information Catalogue and Index Management (p. 109)	capability CT3	Canonical / Established
BC CT3: Health Information Compartmentalisation (p. 110)	capability CT3	Canonical / Established
BC CT3: Health Information Data Correlation (p. 111)	capability CT3	Canonical / Established
BC CT3: Health Information Data Retrieval (p. 112)	capability CT3	Canonical / Established

10.7.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
To: enabled by	BEC CT2: Clinical Record Administration (p. 443)	Proposed
To: enabled by	BEC CT3: Linked Information Curation (p. 663)	Proposed

10.7.8. BC CT3: Health Information Data Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-data-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:654 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.8.1. Capability Overview

Health Information Data Management concerns the actual content maintained within applicable governed structures. It keeps the represented information distinct from definitions describing how that information should be organised and interpreted. Within Health Information Management, its responsibility is meaningful content rather than mere successful persistence of values.

10.7.8.2. Definition / governed semantic detail

Manage actual health-information content within applicable governed structures.

10.7.8.3. Boundary

Managing content does not originate its clinical facts or acquire the authority of its semantic references.

10.7.9. BC CT3: Health Information Structure Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-structure-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-STRUCTURE-MANAGEMENT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:655 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.9.1. Capability Overview

This capability manages the structures, element definitions, references and bindings that make content interpretable. It addresses how a represented item can have an unambiguous meaning without treating the structure as the clinical fact itself. Semantic bindings identify applicable governed references while preserving their external authority.

10.7.9.2. Definition / governed semantic detail

Manage structures, element/reference definitions, constraints and semantic bindings giving content unambiguous meaning.

10.7.9.3. Boundary

Structure management is not terminology governance and does not prescribe FHIR resources, physical schemas or storage technology.

10.7.10. BC CT3: Health Information Lifecycle Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-lifecycle-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-LIFECYCLE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:656 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.10.1. Capability Overview

Health Information Lifecycle Management governs how information's state and temporal standing change through amendment, correction, supersession, retention and disposition. It allows current knowledge to change without retrospectively rewriting what was previously received, known or managed. That distinction supports interpretation of both the present representation and its historical operational meaning.

10.7.10.2. Definition / governed semantic detail

Govern state, amendment, correction, supersession, currency, retention and disposition while preserving provenance and historical meaning.

10.7.10.3. Boundary

Current authoritative state differs from historical truth; preserving either does not confer originating authority for the represented facts.

10.7.11. BC CT3: Health Information Catalogue and Index Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-catalogue-and-index-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-CATALOGUE-AND-INDEX-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:657 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.11.1. Capability Overview

This capability maintains the descriptive aids through which information existence and location can be established. It makes information discoverable without equating the catalogue entry

with the content it describes. Within information management it answers where relevant information may be found before the separate question of obtaining it under an authorised context.

10.7.11.2. Definition / governed semantic detail

Manage descriptive catalogues/indexes through which information existence and location can be established.

10.7.11.3. Boundary

Discoverability does not authorise access, retrieve the content or communicate it to a recipient.

10.7.12. BC CT3: Health Information Compartmentalisation

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-compartmentalisation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-COMPARTMENTALISATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:658 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.12.1. Capability Overview

Health Information Compartmentalisation establishes contextual associations and bounded groupings of information. It makes the relationship of information to a business context explicit, allowing groupings to carry meaning beyond physical storage location. That context can inform governed use while remaining distinct from sensitivity classification and a particular access decision.

10.7.12.2. Definition / governed semantic detail

Manage contextual association, partitioning and bounded information groupings.

10.7.12.3. Boundary

Compartment membership neither establishes permission nor replaces classification or terminology governance.

10.7.13. BC CT3: Health Information Data Correlation

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-data-correlation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-CORRELATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:659 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.13.1. Capability Overview

Health Information Data Correlation establishes meaningful relationships between independently managed information items or sets. It helps information be understood together without requiring a merger of their identities, histories or originating authority. The responsibility is the association itself, rather than a clinical conclusion inferred from the associated content.

10.7.13.2. Definition / governed semantic detail

Establish meaningful relationships between independently managed information items/sets.

10.7.13.3. Boundary — v0.1 explanation

Correlation does not establish person identity, clinical interpretation or ownership transfer between the related items.

10.7.14. BC CT3: Health Information Data Retrieval

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-data-retrieval`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-RETRIEVAL`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Health Information Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:660 (p. 1344); business:650 (p. 1343); business:662 (p. 1344)	Source standing does not approve the derived assertion.

10.7.14.1. Capability Overview

Health Information Data Retrieval obtains identified information under an applicable authorised request and context. It supplies the content whose existence may already be known through a catalogue or index. The distinction matters because locating an item, obtaining it and presenting it to a person each establish different outcomes.

10.7.14.2. Definition / governed semantic detail

Obtain identified information under an applicable authorised request/context.

10.7.14.3. Boundary

Retrieval does not itself grant permission, provide participant communication or confer authority over the retrieved facts.

10.8. BC CT2: Clinical Knowledge Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:664 (p. 1344); business:666 (p. 1344); business:668 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.1. Capability Overview

Clinical Knowledge Management turns potentially useful source material into explicit reusable knowledge with an intelligible basis and governed standing. Context identification and extraction preserve where material came from; formulation expresses applicability and rationale; concept association supplies semantic context; review and publication govern its intended use. These responsibilities distinguish source information, formulated knowledge and approved knowledge without prescribing a compulsory workflow.

10.8.2. Definition / governed semantic detail

Identify, extract, formulate, associate, govern and share reusable clinical knowledge.

10.8.3. Intent

Make reusable clinical knowledge available with the qualifications and approval standing needed to judge its use.

10.8.4. Boundary — v0.1 explanation

Publication does not establish applicability to an individual or a clinical decision. Concept association does not redefine terminology, and formulation does not constitute approval.

10.8.5. Preserved v0.1 Review Matters

Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

10.8.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Knowledge Context Identification (p. 114)	capability CT3	Canonical / Established

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Knowledge Context Extraction (p. 115)	capability CT3	Canonical / Established
BC CT3: Clinical Knowledge Information Formulation (p. 116)	capability CT3	Canonical / Established
BC CT3: Clinical Knowledge Concept Association (p. 116)	capability CT3	Canonical / Established
BC CT3: Clinical Knowledge Review & Approval Management (p. 117)	capability CT3	Canonical / Established
BC CT3: Clinical Knowledge Publication & Sharing (p. 118)	capability CT3	Canonical / Established

10.8.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed
To: enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed

10.8.8. BC CT3: Clinical Knowledge Context Identification

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-context-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Clinical Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:674 (p. 1344); business:670 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.8.1. Capability Overview

This capability identifies sources or contexts that may contain reusable clinical knowledge. It establishes where potentially useful material can be sought without assuming that all source information is already formulated knowledge. Within knowledge management it provides a contextual starting point rather than an approval or publication outcome.

10.8.8.2. Definition / governed semantic detail

Identify sources/contexts potentially containing reusable clinical knowledge.

10.8.8.3. Boundary

Identifying a source does not extract its material, establish reusable knowledge or determine its clinical applicability.

10.8.9. BC CT3: Clinical Knowledge Context Extraction

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-context-extraction`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-EXTRACTION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Clinical Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:675 (p. 1344); business:670 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.9.1. Capability Overview

Clinical Knowledge Context Extraction selects relevant material while retaining its relationship to the source and context. Its purpose is to make potential knowledge material available without stripping away the circumstances needed to understand it. Extracted material remains distinguishable from the reusable knowledge subsequently formulated from it.

10.8.9.2. Definition / governed semantic detail

Extract relevant material while preserving its source/context relationship.

10.8.9.3. Boundary — v0.1 explanation

Extraction does not establish approval or turn source information into an individual clinical conclusion.

10.8.10. BC CT3: Clinical Knowledge Information Formulation

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-information-formulation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-INFORMATION-FORMULATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Clinical Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:676 (p. 1344); business:670 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.10.1. Capability Overview

This capability expresses reusable clinical knowledge with its applicability, qualifications, rationale and evidence relationships. It moves beyond extracted material by making the knowledge claim and its limits explicit. That formulation supports later review and use while retaining the distinction between a proposed knowledge expression and one approved for a governed purpose.

10.8.10.2. Definition / governed semantic detail

Formulate explicit reusable knowledge, including applicability, qualifications, rationale and evidence relationships.

10.8.10.3. Boundary

Formulation does not approve knowledge, govern terminology or define health-information structures.

10.8.11. BC CT3: Clinical Knowledge Concept Association

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-concept-association`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONCEPT-ASSOCIATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Clinical Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:677 (p. 1344); business:670 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.11.1. Capability Overview

Clinical Knowledge Concept Association relates reusable knowledge to the governed clinical concepts relevant to its meaning. It makes the semantic context of the knowledge intelligible without allowing local association to redefine the referenced concepts. The association assists interpretation while preserving the distinction between knowledge authority and concept authority.

10.8.11.2. Definition / governed semantic detail

Associate knowledge with applicable governed clinical concepts without acquiring concept authority.

10.8.11.3. Boundary

Associating knowledge with a terminology concept does not acquire terminology governance or establish approval of the knowledge.

10.8.12. BC CT3: Clinical Knowledge Review & Approval Management

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-review-approval-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-REVIEW-APPROVAL-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Clinical Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:678 (p. 1344); business:670 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.12.1. Capability Overview

This capability manages the review and governed standing of reusable knowledge, including endorsement, rejection, conditions and history. It establishes how the knowledge may legitimately be treated for its intended use rather than assuming that formulation makes it trustworthy or approved. Temporal standing matters where a later review changes the conditions for use.

10.8.12.2. Definition / governed semantic detail

Manage review, endorsement, approval/rejection, conditions and applicable history for intended governed use.

10.8.12.3. Boundary — v0.1 explanation

Approval does not establish applicability to a particular individual or replace the responsible clinical judgement.

10.8.13. BC CT3: Clinical Knowledge Publication & Sharing

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-knowledge-publication-sharing`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-PUBLICATION-SHARING`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Information & Knowledge Management → BC CT2: Clinical Knowledge Management	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:679 (p. 1344); business:670 (p. 1344); business:681 (p. 1344)	Source standing does not approve the derived assertion.

10.8.13.1. Capability Overview

Clinical Knowledge Publication & Sharing makes approved knowledge available to applicable authorised consumers and contexts. It preserves the standing and limits under which knowledge was approved rather than reducing publication to unrestricted dissemination. Its contribution is availability for appropriate use, distinct from both approval and the individual clinical application that may follow.

10.8.13.2. Definition / governed semantic detail

Make approved knowledge available to applicable authorised consumers and use contexts.

10.8.13.3. Boundary

Published knowledge is not a patient-specific conclusion, decision or instruction merely because a clinician can access it.

11. BC CT1: Standards, Semantics & Reference Governance

Property	Value	Meaning
Reference Id	HCM-BC-standards-semantics-reference-governance	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:683 (p. 1345); business:687 (p. 1345); business:120 (p. 1332)	Source standing does not approve the derived assertion.

11.1. Capability Overview

This capability establishes the organisational standing and appropriate use of models, standards and semantic artefacts. Standards governance addresses discovery, adoption, education, compliance and impacts; clinical-semantics governance addresses terminology, classifications and mappings. It provides the governed meaning within which information can be represented and exchanged, rather than merely maintaining technical schemas.

11.2. Definition / governed semantic detail

Govern organisational adoption, oversight and appropriate use of health-information models, standards, terminology, classifications and semantic mappings.

11.3. Intent

Support consistent, appropriate interpretation and use of health-information standards and semantic references.

11.4. Boundary

Using a terminology binding, mapping or classification does not transfer its governing authority. The established peer set must not be expanded simply to create a symmetrical taxonomy.

11.5. Harmonia Architectural Relevance

Semantic and conformance governance is a substantive platform responsibility. Its organisational meaning constrains technical representations; implementation products do not define that meaning.

11.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Health Information Standards Governance (p. 121)	capability CT2	Canonical / Established
BC CT2: Clinical Semantics Governance (p. 128)	capability CT2	Canonical / Established

11.7. BC CT2: Health Information Standards Governance

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-standards-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:691 (p. 1345); business:693 (p. 1345); business:695 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.1. Capability Overview

This capability governs which information models and standards the organisation adopts and how their use is supported and evaluated. The model distinguishes discovering candidates, adopting models, educating users, addressing conformity and overseeing governance. Standards impact identification has a separate established place because its implications may extend into clinical practice, policy and obligations beyond information-system implementation.

11.7.2. Definition / governed semantic detail

Govern organisational adoption and use of health-information models and applicable standards.

11.7.3. Intent

Make organisational standards use deliberate and its consequences understandable.

11.7.4. Boundary — v0.1 explanation

Oversight evaluates governance without performing every peer activity. Adoption includes its own model-impact consideration; the listed abilities do not establish a sequential adoption Process.

11.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Information Model Discovery (p. 122)	capability CT3	Canonical / Established
BC CT3: Information Model Adoption (p. 123)	capability CT3	Canonical / Established
BC CT3: Information Model Education (p. 124)	capability CT3	Canonical / Established
BC CT3: Information Model Compliance (p. 125)	capability CT3	Canonical / Established
BC CT3: Information Model Governance Oversight (p. 125)	capability CT3	Canonical / Established
BC CT3: Standards Adoption (p. 126)	capability CT3	Canonical / Established
BC CT3: Standards Impact Identification (p. 127)	capability CT3	Canonical / Established

11.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Information Design Governance (p. 792)	Proposed

11.7.7. BC CT3: Information Model Discovery

Property	Value	Meaning
Reference Id	`HCM-BC-information-model-discovery`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-DISCOVERY`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:701 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.7.1. Capability Overview

Information Model Discovery identifies candidate models relevant to the organisation's health-information needs. It supports informed standards governance by making applicable possibilities visible before adoption. Its business purpose is finding suitable semantic frames rather than selecting a software representation or assuming every discovered model should be used.

11.7.7.2. Definition / governed semantic detail

Identify information models relevant to the organisation's health-information needs.

11.7.7.3. Boundary

Discovery does not adopt a model, establish compliance or grant it organisational authority.

11.7.8. BC CT3: Information Model Adoption

Property	Value	Meaning
Reference Id	`HCM-BC-information-model-adoption`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-ADOPTION`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:702 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.8.1. Capability Overview

Information Model Adoption establishes the organisation's use of an applicable information model, including consideration of the consequences of adopting it. It gives a discovered model an organisational standing distinct from mere availability. The intrinsic impact consideration helps adoption remain a business governance responsibility rather than a mechanical import of definitions.

11.7.8.2. Definition / governed semantic detail

Adopt applicable information models, including intrinsic assessment of adoption impacts.

11.7.8.3. Boundary

Adoption does not perform every education, compliance or governance-oversight responsibility associated with subsequent use.

11.7.9. BC CT3: Information Model Education

Property	Value	Meaning
Reference Id	`HCM-BC-information-model-education`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-EDUCATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:703 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.9.1. Capability Overview

Information Model Education supports understanding and appropriate use of adopted models. It addresses the people and practice dimension of semantic consistency: a published model is not necessarily a model understood in the contexts where it is applied. Its responsibility therefore complements adoption and compliance without becoming either.

11.7.9.2. Definition / governed semantic detail

Support organisational understanding and appropriate use of adopted information models.

11.7.9.3. Boundary

Educational support neither changes the authoritative model nor establishes that actual use complies with it.

11.7.10. BC CT3: Information Model Compliance

Property	Value	Meaning
Reference Id	`HCM-BC-information-model-compliance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-COMPLIANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:704 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.10.1. Capability Overview

Information Model Compliance addresses conformity to applicable model requirements. It makes actual organisational use accountable to the adopted semantic frame rather than treating model availability or user education as proof of conformity. Within standards governance it concerns the requirements for use, not the separate task of discovering or adopting models.

11.7.10.2. Definition / governed semantic detail

Address organisational conformity to applicable information-model requirements.

11.7.10.3. Boundary

Conformity does not redefine the model or absorb governance oversight of the wider adoption arrangements.

11.7.11. BC CT3: Information Model Governance Oversight

Property	Value	Meaning
Reference Id	`HCM-BC-information-model-governance-oversight`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-GOVERNANCE-OVERSIGHT`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:705 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.11.1. Capability Overview

This capability evaluates the effectiveness and consistency of information-model governance itself. It asks whether discovery, adoption and use are being governed appropriately, rather than repeating each operational peer. Its purpose is coherent oversight of the arrangements that give models standing and guide their application.

11.7.11.2. Definition / governed semantic detail

Oversee and evaluate effectiveness, consistency and appropriate operation of information-model governance.

11.7.11.3. Boundary

Oversight does not itself perform discovery, adoption, education or compliance activities.

11.7.12. BC CT3: Standards Adoption

Property	Value	Meaning
Reference Id	`HCM-BC-standards-adoption`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-STANDARDS-ADOPTION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:706 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.12.1. Capability Overview

Standards Adoption establishes which applicable standards the organisation adopts. It gives standards an intentional place in organisational practice rather than allowing familiar products or conventions to define requirements by default. The business responsibility concerns adoption in context, including standards whose effects extend beyond information representation.

11.7.12.2. Definition / governed semantic detail

Adopt applicable standards within the organisation.

11.7.12.3. Boundary — v0.1 explanation

Adoption does not establish a complete standards lifecycle taxonomy or perform the distinct identification of broader standards impacts.

11.7.13. BC CT3: Standards Impact Identification

Property	Value	Meaning
Reference Id	`HCM-BC-standards-impact-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-STANDARDS-IMPACT-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Health Information Standards Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:707 (p. 1345); business:697 (p. 1345); business:709 (p. 1345)	Source standing does not approve the derived assertion.

11.7.13.1. Capability Overview

Standards Impact Identification recognises the consequences of standards for relevant organisational activity. Impacts may concern clinical practice, medications, procedures, policy, safety, obligations or reporting, so the responsibility cannot be reduced to technical compatibility. It makes those consequences visible for the responsible capabilities to consider within their own authority.

11.7.13.2. Definition / governed semantic detail

Identify standards impacts across clinical/work practice, permitted medications, procedures, policy, safety, obligations and reporting as applicable.

11.7.13.3. Boundary

Identifying an impact neither adopts the standard nor acquires the operational authority needed to respond to it.

11.8. BC CT2: Clinical Semantics Governance

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-semantics-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:711 (p. 1345); business:713 (p. 1345); business:715 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.1. Capability Overview

Clinical Semantics Governance maintains the standing and appropriate use of terminology, classifications and semantic mappings. Terminology provides consistent meaning; classification concerns sensitivity and consequences of exposure or mishandling. The established children distinguish governing artefacts from overseeing actual use and permit controlled terminology localisation where adopted material does not meet legitimate needs.

11.8.2. Definition / governed semantic detail

Govern clinical meaning and appropriate organisational use of semantic artefacts.

11.8.3. Intent

Keep meaning and classification intelligible across organisational use without concealing semantic drift or local variation.

11.8.4. Boundary

Compartmentalisation concerns contextual relationships rather than classification or terminology. Classification Localisation is not an established peer; mappings must preserve the separate meanings and authorities they connect.

11.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Terminology Governance (p. 129)	capability CT3	Canonical / Established
BC CT3: Terminology Utilisation Oversight (p. 130)	capability CT3	Canonical / Established
BC CT3: Terminology Localisation (p. 131)	capability CT3	Canonical / Established
BC CT3: Classification Governance (p. 132)	capability CT3	Canonical / Established
BC CT3: Classification Utilisation Oversight (p. 133)	capability CT3	Canonical / Established
BC CT3: Semantic Mapping Governance (p. 133)	capability CT3	Canonical / Established

11.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Information Design Governance (p. 792)	Proposed
To: enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed

11.8.7. BC CT3: Terminology Governance

Property	Value	Meaning
Reference Id	`HCM-BC-terminology-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-GOVERNANCE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Clinical Semantics Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:721 (p. 1345); business:717 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.7.1. Capability Overview

Terminology Governance maintains the lifecycle and standing of terminology and reference sets used by the organisation. Acceptance, release, updates and retirement require coordination with the appropriate authority or provider so that a code's meaning is not treated as timeless or locally self-defined. This supplies a governed semantic basis for information bindings and knowledge associations.

11.8.7.2. Definition / governed semantic detail

Manage governed terminology/reference-set lifecycle and standing, including acceptance, release, update, supersession/retirement and authority/provider coordination.

11.8.7.3. Boundary

Governing terminology differs from binding a structure to it or overseeing its actual use.

11.8.8. BC CT3: Terminology Utilisation Oversight

Property	Value	Meaning
Reference Id	`HCM-BC-terminology-utilisation-oversight`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-UTILISATION-OVERSIGHT`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Clinical Semantics Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:722 (p. 1345); business:717 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.8.1. Capability Overview

Terminology Utilisation Oversight considers how terminology and reference sets are actually used. It identifies inappropriate application, inconsistency or semantic drift that the existence of an approved terminology does not prevent. Its purpose is to preserve coherent use without making each consuming capability the owner of terminology meaning.

11.8.8.2. Definition / governed semantic detail

Oversee actual terminology/reference-set use and identify inappropriate, inconsistent or drifting usage.

11.8.8.3. Boundary

Use oversight does not redefine terminology or automatically authorise a local semantic variation.

11.8.9. BC CT3: Terminology Localisation

Property	Value	Meaning
Reference Id	`HCM-BC-terminology-localisation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-LOCALISATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Clinical Semantics Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:723 (p. 1345); business:717 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.9.1. Capability Overview

Terminology Localisation manages controlled local responses where adopted terminology does not satisfy legitimate organisational semantic needs. It makes the local variation explicit rather than allowing ad hoc usage to conceal a gap or change in meaning. Within clinical-semantics governance, this preserves a distinction between governed local adaptation and uncontrolled drift.

11.8.9.2. Definition / governed semantic detail

Manage controlled local responses where adopted terminology does not meet legitimate organisational semantic requirements.

11.8.9.3. Boundary

The established localisation responsibility concerns terminology; no corresponding Classification Localisation capability is inferred.

11.8.10. BC CT3: Classification Governance

Property	Value	Meaning
Reference Id	`HCM-BC-classification-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-CLASSIFICATION-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Clinical Semantics Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:724 (p. 1345); business:717 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.10.1. Capability Overview

Classification Governance establishes the definitions and organisational standing of applicable classifications. In this governed context, classification concerns sensitivity and the consequences of exposure or mishandling, rather than the clinical meaning supplied by terminology. Its outcome supplies a basis for consistent application and protection considerations.

11.8.10.2. Definition / governed semantic detail

Govern applicable classification definitions and their organisational standing.

11.8.10.3. Boundary

Classification is orthogonal to contextual compartment membership and does not by itself determine an access decision.

11.8.11. BC CT3: Classification Utilisation Oversight

Property	Value	Meaning
Reference Id	`HCM-BC-classification-utilisation-oversight`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-CLASSIFICATION-UTILISATION-OVERSIGHT`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Clinical Semantics Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:725 (p. 1345); business:717 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.11.1. Capability Overview

Classification Utilisation Oversight evaluates appropriate organisational application of classifications. It addresses whether information is treated consistently with the governed sensitivity meaning, rather than assuming that an established classification scheme guarantees correct use. This retains a distinct responsibility for practice alongside definition governance.

11.8.11.2. Definition / governed semantic detail

Oversee appropriate organisational application of classifications.

11.8.11.3. Boundary

Oversight neither creates classification definitions nor establishes a new localisation peer.

11.8.12. BC CT3: Semantic Mapping Governance

Property	Value	Meaning
Reference Id	`HCM-BC-semantic-mapping-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-SEMANTIC-MAPPING-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Standards, Semantics & Reference Governance → BC CT2: Clinical Semantics Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:726 (p. 1345); business:717 (p. 1345); business:728 (p. 1346)	Source standing does not approve the derived assertion.

11.8.12.1. Capability Overview

Semantic Mapping Governance maintains mappings between applicable semantic artefacts while preserving the meaning and authority of each side. Its purpose is intelligible correspondence rather than assumed equivalence. A governed mapping can support exchange or interpretation without suggesting that the mapped artefacts have become the same concept or share one authority.

11.8.12.2. Definition / governed semantic detail

Govern mappings between applicable semantic artefacts without collapsing their source meanings or authorities.

11.8.12.3. Boundary

A mapping does not erase source distinctions or give a consuming system authority to redefine either source.

12. BC CT1: Connected Health Services

Property	Value	Meaning
Reference Id	`HCM-BC-connected-health-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:730 (p. 1346); business:734 (p. 1346); business:120 (p. 1332)	Source standing does not approve the derived assertion.

12.1. Capability Overview

Connected Health Services represents the organisation's ability to exchange health information and communicate digitally between health-service participants. Its two children distinguish exchange of health-information artefacts from participant communication, notification and collaboration. Both support connected care, but a user viewing information through a screen is not thereby participating in a communication channel.

12.2. Definition / governed semantic detail

Exchange health information appropriately and provide digitally mediated communication, notifications and collaborative interaction between health-service participants.

12.3. Intent

Enable appropriate information movement and interaction between otherwise separate participants and services.

12.4. Boundary

This organisational capability is not defined by an interface engine or messaging product. General presentation remains a separate enabling concern.

12.5. Harmonia Architectural Relevance

Information exchange and digitally mediated interaction are direct platform responsibilities within their assigned contracts, supporting the Core classification without transferring payload or clinical authority.

12.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Health Information Exchange (p. 136)	capability CT2	Canonical / Established
BC CT2: Digital Communication Channel (p. 138)	capability CT2	Canonical / Established

12.7. BC CT2: Health Information Exchange

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-exchange`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES.CT2-HEALTH-INFORMATION-EXCHANGE`	Current proposed context where marked Proposed.
Current context	BC CT1: Connected Health Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:738 (p. 1346); business:740 (p. 1346); business:742 (p. 1346); business:746 (p. 1346); business:1225 (p. 1358)	Source standing does not approve the derived assertion.

12.7.1. Capability Overview

As a Business Capability, Health Information Exchange is the organisational ability to move health information appropriately between participating parties, systems and services. It explains the enterprise purpose served by exchange without defining that purpose as a protocol, server or software product. Its placement under Connected Health Services distinguishes information exchange from the broader interaction and notification purposes of Digital Communication Channel.

12.7.2. Definition / governed semantic detail

Exchange health information appropriately between participating parties, systems and services.

12.7.3. Boundary — v0.1 explanation

No CT3 decomposition is established. The identically named BEC is a distinct enabling responsibility; the naming conflict remains unresolved, and this entry does not merge the two or assign the sharing-programme concern a parent.

12.7.4. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Information Sharing Programme Management (p. 137)	capability CT3	Candidate / Proposed

12.7.5. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Exchange Enablement (p. 686)	Proposed

12.7.6. BC CT3: Health Information Sharing Programme Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-sharing-programme-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Candidate	Preserved Candidate concern; not promoted.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES.CT2-HEALTH-INFORMATION-EXCHANGE.CT3-BC-HEALTH-INFORMATION-SHARING-PROGRAMME-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Connected Health Services → BC CT2: Health Information Exchange	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1222 (p. 1358)	Source standing does not approve the derived assertion.

12.7.6.1. Capability Overview

Programme-level coordination supplies a plausible frame for the preserved name without replacing the established exchange abilities.

12.7.6.2. Semantic elaboration — Proposed

Coordinate the governed scope, participants and change context of a health-information-sharing programme.

12.7.6.3. Additional Notes / Unresolved Matters — v0.1 explanation

Definition, tier and parent remain unresolved. This is an explicitly constrained description, not an approved programme-management responsibility or an allocated structural path.

12.7.6.4. Boundary

The element remains Candidate. The definition/placement elaboration is Proposed and does not adjudicate the unresolved RAD question or transfer source-domain authority.

12.7.6.5. Alternative Considered / Review Note

Framework oversight alone is too narrow; programme authority and exact boundary require review.

12.7.6.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

12.7.6.7. Preserved v0.1 Review Matters

DN-BC-027 establishes spelling of this named working concern, not a precise tier/parent or definition.

12.8. BC CT2: Digital Communication Channel

Property	Value	Meaning
Reference Id	`HCM-BC-digital-communication-channel`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES.CT2-DIGITAL-COMMUNICATION-CHANNEL`	Current proposed context where marked Proposed.
Current context	BC CT1: Connected Health Services	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:748 (p. 1346); business:750 (p. 1346); business:752 (p. 1346); business:756 (p. 1346)	Source standing does not approve the derived assertion.

12.8.1. Capability Overview

Digital Communication Channel supports communication between patients, practitioners and other participants through digitally mediated messages, notifications and collaborative interaction. Its organisational purpose is the exchange between participants: for example, a notification can connect someone to a care event without itself delivering care or changing the event's originating facts.

12.8.2. Definition / governed semantic detail

Provide digitally mediated communication, notification and collaborative interaction between patients, practitioners and other health-service participants.

12.8.3. Boundary

A presentation interface used to exercise another capability is not automatically this communication ability. No CT3 set or general presentation BEC placement is established here.

12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Clinical Communication Administration (p. 476)	Proposed
To: enabled by	BEC CT2: General Collaboration (p. 768)	Proposed

13. BC CT1: Security, Privacy & Digital Trust

Property	Value	Meaning
Reference Id	`HCM-BC-security-privacy-digital-trust`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:758 (p. 1346); business:762 (p. 1346); business:120 (p. 1332)	Source standing does not approve the derived assertion.

13.1. Capability Overview

This capability establishes confidence in identity, coherent control of access, protection of information and accountability for its use. Its three branches distinguish identity/access governance, protection and privacy, and attribution/accountability. The scope includes physical and procedural arrangements as well as digital controls, preventing a software authorisation check from standing in for the organisation's whole trust responsibility.

13.2. Definition / governed semantic detail

Establish adequate identity confidence, coherent access controls, privacy/protection safeguards, governed activity attribution and accountability for security-relevant activity and information use.

13.3. Intent

Support information use and governed activity within legitimate identity, authority, privacy and protection constraints.

13.4. Boundary

Identifying an actor, permitting an action, recording participation and judging appropriateness are different outcomes. Evidence production is not itself independent assurance.

13.5. Harmonia Architectural Relevance

Security and governance are intrinsic to Harmonia-managed operations. The Core classification concerns assigned platform controls and evidence; it does not acquire every organisational security or privacy responsibility.

13.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Digital Identity & Access Governance (p. 141)	capability CT2	Canonical / Established
BC CT2: Health Information Protection (p. 149)	capability CT2	Canonical / Established
BC CT2: Digital Accountability & Trust (p. 155)	capability CT2	Canonical / Established

13.7. BC CT2: Digital Identity & Access Governance

Property	Value	Meaning
Reference Id	`HCM-BC-digital-identity-access-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:766 (p. 1347); business:768 (p. 1347); business:770 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.1. Capability Overview

This capability governs who may act and how controlled concerns are identified and protected across the organisation. Its children separate confidence in a claimed identity, identity models, contextual identity quality, authentication, access-control context, assurance, collaboration and enforcement. That separation prevents successful authentication from being treated as sufficient authority for every use.

13.7.2. Definition / governed semantic detail

Govern sufficient identity confidence and coherent access controls across the organisation.

13.7.3. Intent

Make identity confidence and control requirements appropriate to the circumstances in which access or activity occurs.

13.7.4. Boundary

Federation or Single Sign-On does not guarantee identity quality. Enforcement spans physical, procedural and digital controls; it is broader than a runtime policy engine.

13.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Identity Assurance (p. 142)	capability CT3	Canonical / Established
BC CT3: Identity Modelling & Interoperability (p. 143)	capability CT3	Canonical / Established
BC CT3: Identity Qualification Control (p. 144)	capability CT3	Canonical / Established
BC CT3: Identity Identification & Authentication (p. 145)	capability CT3	Canonical / Established
BC CT3: Access Control Context Management (p. 146)	capability CT3	Canonical / Established
BC CT3: Access Control Assurance & Reporting (p. 147)	capability CT3	Canonical / Established
BC CT3: Access Control Collaboration (p. 147)	capability CT3	Canonical / Established
BC CT3: Access Control Enforcement (p. 148)	capability CT3	Canonical / Established

13.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Health Information Control (p. 715)	Proposed
To: enabled by	BEC CT3: Client Privacy Guardrail Management (p. 237)	Proposed

13.7.7. BC CT3: Identity Assurance

Property	Value	Meaning
Reference Id	`HCM-BC-identity-assurance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-ASSURANCE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:776 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.7.1. Capability Overview

Identity Assurance establishes confidence that a person is who they claim to be using appropriate evidence and processes. It concerns the basis and strength of the identity claim, rather than only successful use of an authentication mechanism. Within identity governance it makes confidence a question that can be considered separately from contextual sufficiency and access authority.

13.7.7.2. Definition / governed semantic detail

Establish sufficient confidence that a person is who they claim to be through appropriate evidence and processes.

13.7.7.3. Boundary

An assured identity is not automatically sufficient for every context or permitted to perform every action.

13.7.8. BC CT3: Identity Modelling & Interoperability

Property	Value	Meaning
Reference Id	`HCM-BC-identity-modelling-interoperability`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-MODELLING-INTEROPERABILITY`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:777 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.8.1. Capability Overview

This capability establishes identity models and arrangements through which identities can be understood across organisational use. It addresses the coherence needed when parties exchange or rely on identity information without assuming that their identity assertions have identical quality. Its purpose is meaningful interoperability, rather than a particular federation or Single Sign-On technology.

13.7.8.2. Definition / governed semantic detail

Establish identity models and interoperability arrangements appropriate to organisational identity use.

13.7.8.3. Boundary

Interoperability does not guarantee assurance quality or transfer each party's control ownership.

13.7.9. BC CT3: Identity Qualification Control

Property	Value	Meaning
Reference Id	`HCM-BC-identity-qualification-control`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-QUALIFICATION-CONTROL`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:778 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.9.1. Capability Overview

Identity Qualification Control evaluates whether presented identity characteristics, provenance and assurance are sufficient for the intended context. It adds a contextual judgement beyond

whether the actor can be authenticated. This permits an identity adequate for one purpose to remain insufficient for another without claiming that the identity is necessarily false.

13.7.9.2. Definition / governed semantic detail

Determine whether presented identity characteristics, provenance and assurance quality are sufficient for its intended context.

13.7.9.3. Boundary — v0.1 explanation

Identity qualification does not establish practitioner clinical qualification or a general permission to act.

13.7.10. BC CT3: Identity Identification & Authentication

Property	Value	Meaning
Reference Id	`HCM-BC-identity-identification-authentication`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-IDENTIFICATION-AUTHENTICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:779 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.10.1. Capability Overview

This capability identifies the acting participant and authenticates the asserted identity. It establishes who is acting under the applicable identity arrangements so that later control and accountability can be grounded in an identified actor. The outcome remains distinct from confidence established through identity evidence and from authority to access a controlled concern.

13.7.10.2. Definition / governed semantic detail

Identify an actor and authenticate the asserted identity.

13.7.10.3. Boundary

Authentication neither establishes every required identity-quality condition nor authorises the requested activity.

13.7.11. BC CT3: Access Control Context Management

Property	Value	Meaning
Reference Id	`HCM-BC-access-control-context-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-CONTEXT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:780 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.11.1. Capability Overview

Access Control Context Management establishes what requires control, the circumstances in which control applies and how those requirements are modelled. It gives access governance an explicit subject and context rather than reducing it to assembling runtime attributes. The responsibility can encompass information, functions and activities under physical, procedural or digital arrangements.

13.7.11.2. Definition / governed semantic detail

Determine what information, functions, activities or other concerns require control, under what circumstances and how requirements are modelled.

13.7.11.3. Boundary

Defining control context does not itself enforce the controls or establish that they operate effectively.

13.7.12. BC CT3: Access Control Assurance & Reporting

Property	Value	Meaning
Reference Id	`HCM-BC-access-control-assurance-reporting`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ASSURANCE-REPORTING`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:781 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.12.1. Capability Overview

This capability establishes organisational evidence and assurance about whether access-control arrangements are appropriately established and operating. It makes control effectiveness a distinct concern from defining requirements or performing an individual enforcement action. Reporting communicates that established position without making an isolated successful check proof of end-to-end control.

13.7.12.2. Definition / governed semantic detail

Establish organisational assurance/evidence of whether access-control arrangements are appropriately established and operating.

13.7.12.3. Boundary

Subject-specific access-control assurance does not create another macro-level Health Service Assurance hierarchy.

13.7.13. BC CT3: Access Control Collaboration

Property	Value	Meaning
Reference Id	`HCM-BC-access-control-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:782 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.13.1. Capability Overview

Access Control Collaboration coordinates arrangements across parties or domains that retain independent control ownership. Its purpose is coherent cross-boundary control where a strong local arrangement alone cannot establish the whole result. Collaboration therefore concerns agreed control participation rather than silently centralising every participant's authority.

13.7.13.2. Definition / governed semantic detail

Coordinate arrangements across independently controlled domains/parties while retaining their control ownership.

13.7.13.3. Boundary

Coordination does not transfer the parties' control ownership or guarantee identity quality across domains.

13.7.14. BC CT3: Access Control Enforcement

Property	Value	Meaning
Reference Id	`HCM-BC-access-control-enforcement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ENFORCEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Identity & Access Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:783 (p. 1347); business:772 (p. 1347); business:785 (p. 1347)	Source standing does not approve the derived assertion.

13.7.14.1. Capability Overview

Access Control Enforcement ensures that intended controls are coherently imposed through the applicable physical, procedural and digital mechanisms. It represents the organisational ability to make controls effective, rather than equating enforcement with one software decision. Its outcome depends on the controlled circumstances and the arrangements intended to apply there.

13.7.14.2. Definition / governed semantic detail

Ensure physical, procedural and digital control mechanisms coherently impose intended controls.

13.7.14.3. Boundary — v0.1 explanation

A local software authorisation result alone does not establish coherent end-to-end organisational enforcement.

13.8. BC CT2: Health Information Protection

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-protection`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:787 (p. 1347); business:789 (p. 1347); business:791 (p. 1347); business:803 (p. 1348)	Source standing does not approve the derived assertion.

13.8.1. Capability Overview

Health Information Protection governs the consequences of information exposure and the safeguards protecting information and its operating environment. Privacy safeguards, estate protection, access-practice monitoring, compartment assurance and classification assurance address different questions. Together they distinguish appropriate arrangements from evidence of how information is actually handled, including physical and printed handling.

13.8.2. Definition / governed semantic detail

Govern privacy consequences and adequate protection of the information estate and its environment.

13.8.3. Intent

Protect information and the people affected by its use, with an intelligible basis for evaluating protective arrangements.

13.8.4. Boundary

Compartment assurance does not decide access. Subject-specific protection assurance does not create additional macro-level Health Service Assurance trees.

13.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Privacy Safeguard Management (p. 150)	capability CT3	Canonical / Established
BC CT3: Health Information Safeguarding (p. 151)	capability CT3	Canonical / Established
BC CT3: Health Information Access Monitoring (p. 152)	capability CT3	Canonical / Established
BC CT3: Health Information Compartment Assurance (p. 153)	capability CT3	Canonical / Established
BC CT3: Health Information Classification Assurance (p. 154)	capability CT3	Canonical / Established

13.8.6. BC CT3: Health Privacy Safeguard Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-privacy-safeguard-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-PRIVACY-SAFEGUARD-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Health Information Protection	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:797 (p. 1347); business:793 (p. 1347); business:803 (p. 1348)	Source standing does not approve the derived assertion.

13.8.6.1. Capability Overview

This capability establishes safeguards and assurance arrangements concerning the privacy consequences of supplying, exposing or using information. It focuses on the effect on people and legitimate information use, distinguishing that concern from protection of the information estate itself. Technical access restrictions may contribute without exhausting the organisational privacy responsibility.

13.8.6.2. Definition / governed semantic detail

Establish organisational safeguards and assurance processes concerning privacy consequences of information supply, exposure or use.

13.8.6.3. Boundary

Privacy safeguards do not replace broader estate resilience, access decisions or originating consent authority.

13.8.7. BC CT3: Health Information Safeguarding

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-safeguarding`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-SAFEGUARDING`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Health Information Protection	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:798 (p. 1347); business:793 (p. 1347); business:803 (p. 1348)	Source standing does not approve the derived assertion.

13.8.7.1. Capability Overview

Health Information Safeguarding protects information and its operational environment against loss, compromise, inappropriate access and operational failure. It frames resilience and protection as organisational outcomes, allowing physical controls, recovery arrangements and testing to contribute without defining the capability as a product. The concern is broader than any individual access transaction.

13.8.7.2. Definition / governed semantic detail

Ensure information and its operational environment are protected and resilient against loss, compromise, inappropriate access or operational failure.

13.8.7.3. Boundary

Estate protection remains distinct from evaluating privacy consequences and from assurance conclusions about service delivery.

13.8.8. BC CT3: Health Information Access Monitoring

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-access-monitoring`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-ACCESS-MONITORING`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Health Information Protection	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:799 (p. 1347); business:793 (p. 1347); business:803 (p. 1348)	Source standing does not approve the derived assertion.

13.8.8.1. Capability Overview

Health Information Access Monitoring evaluates actual access and handling practice across organisational circumstances. Digital audit information can contribute, but so can physical access, printed handling, compartments and classification practice. Its purpose is to understand what is happening in use rather than merely observe software events or assume configured controls describe reality.

13.8.8.2. Definition / governed semantic detail

Evaluate actual organisational access/handling practice, including controls, accessibility, compartments, classification and physical/printed handling.

13.8.8.3. Boundary

Evidence supply does not transfer business accountability, and observation is not automatically independent assurance.

13.8.9. BC CT3: Health Information Compartment Assurance

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-compartment-assurance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-COMPARTMENT-ASSURANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Health Information Protection	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:800 (p. 1347); business:793 (p. 1347); business:803 (p. 1348)	Source standing does not approve the derived assertion.

13.8.9.1. Capability Overview

This capability evaluates whether contextual information compartments are appropriately established and applied. It treats the correctness of the grouping as a concern distinct from permission to disclose a particular item. Such assurance can make compartment use more trustworthy without changing the business relationships that define membership.

13.8.9.2. Definition / governed semantic detail

Assure appropriate establishment and application of compartmentalisation without establishing access decisions.

13.8.9.3. Boundary — v0.1 explanation

Assuring compartmentalisation does not establish access decisions or sensitivity classification.

13.8.10. BC CT3: Health Information Classification Assurance

Property	Value	Meaning
Reference Id	`HCM-BC-health-information-classification-assurance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-CLASSIFICATION-ASSURANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Health Information Protection	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:801 (p. 1347); business:793 (p. 1347); business:803 (p. 1348)	Source standing does not approve the derived assertion.

13.8.10.1. Capability Overview

This capability evaluates whether classification is appropriate to information sensitivity and the consequences of exposure or mishandling. It examines the application of classification rather than merely the presence of a label. The responsibility supports protection decisions while

retaining the distinction between the classification basis and the control applied to a particular request.

13.8.10.2. Definition / governed semantic detail

Assure appropriate classification according to sensitivity and potential consequences of exposure/mishandling.

13.8.10.3. Boundary

Classification assurance does not define compartment membership or grant permission to access classified information.

13.9. BC CT2: Digital Accountability & Trust

Property	Value	Meaning
Reference Id	`HCM-BC-digital-accountability-trust`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:805 (p. 1348); business:807 (p. 1348); business:809 (p. 1348); business:818 (p. 1348)	Source standing does not approve the derived assertion.

13.9.1. Capability Overview

Digital Accountability & Trust makes activity attributable and evaluates whether security-relevant activity and information use were appropriate. Attribution records participation together with acting capacity and relevant permission or constraint context; accountability uses that context to consider conduct against policy and obligations. A record of who acted is therefore necessary context, rather than proof that the action was permitted.

13.9.2. Definition / governed semantic detail

Establish governed attribution and accountability for activity and information use.

13.9.3. Intent

Support meaningful responsibility for information use and security-relevant conduct.

13.9.4. Boundary — v0.1 explanation

Authentication, attribution and authorisation must remain distinguishable. Expected downstream evidentiary use does not establish new BC-to-BEC relationships.

13.9.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Governed Activity Attribution (p. 156)	capability CT3	Canonical / Established
BC CT3: Security & Information-Use Accountability (p. 157)	capability CT3	Canonical / Established

13.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT3: Data Provenance Capture (p. 733)	Proposed

13.9.7. BC CT3: Governed Activity Attribution

Property	Value	Meaning
Reference Id	`HCM-BC-governed-activity-attribution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-GOVERNED-ACTIVITY-ATTRIBUTION`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Accountability & Trust	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:815 (p. 1348); business:811 (p. 1348); business:818 (p. 1348)	Source standing does not approve the derived assertion.

13.9.7.1. Capability Overview

Governed Activity Attribution preserves who participated and in what acting capacity, together with the relevant permission, authority and constraint context. It makes conduct explainable

beyond a bare actor identifier and timestamp. Within Digital Accountability & Trust, this supplies the contextual record needed to distinguish participation from entitlement.

13.9.7.2. Definition / governed semantic detail

Preserve who participated, what was done, the attributable person/role/unit/organisation capacity and applicable permission/authority/constraint context.

13.9.7.3. Boundary — v0.1 explanation

Attributing an action to a person or role does not establish that the action was permitted or appropriate.

13.9.8. BC CT3: Security & Information-Use Accountability

Property	Value	Meaning
Reference Id	`HCM-BC-security-information-use-accountability`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-SECURITY-INFORMATION-USE-ACCOUNTABILITY`	Current proposed context where marked Proposed.
Current context	BC CT1: Security, Privacy & Digital Trust → BC CT2: Digital Accountability & Trust	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:816 (p. 1348); business:811 (p. 1348); business:818 (p. 1348)	Source standing does not approve the derived assertion.

13.9.8.1. Capability Overview

This capability evaluates attributed activity and information use against applicable policy, obligations and authority constraints. It establishes accountability rather than simply recording participation. Its purpose depends on preserving the circumstances of action so that appropriateness is considered within the relevant mandate instead of inferred from identity or technical success.

13.9.8.2. Definition / governed semantic detail

Evaluate whether attributed security-relevant activity/information use was appropriate within policy, obligation and authority constraints, and establish accountability.

13.9.8.3. Boundary

An authenticated actor and a successful operation are not proof of legitimate information use.

14. BC CT1: Health Research & Innovation

Property	Value	Meaning
Reference Id	`HCM-BC-health-research-innovation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:824 (p. 1348); business:828 (p. 1348); business:120 (p. 1332)	Source standing does not approve the derived assertion.

14.1. Capability Overview

This capability frames the deliberately bounded research and innovation responsibilities material to Harmonia's R1/R2 architecture. Research Information Support concerns provision of existing information for authorised research; Health Innovation concerns evaluation of proposed or emerging innovation. These establish useful enterprise context without implying a complete research organisation or innovation lifecycle.

14.2. Definition / governed semantic detail

Provide existing health information for authorised research under applicable constraints and evaluate health innovation; deliberately under-decomposed for R1/R2.

14.3. Intent

Support legitimate research information needs and informed consideration of changes to health-service or clinical practice.

14.4. Boundary

Information provision does not own trials, ethics approval or specialist research analysis. Evaluating innovation does not authorise adoption or implementation.

14.5. Harmonia Architectural Relevance

Governed provision of existing information materially enables authorised research. Innovation evaluation has no separate platform assignment; the CT1 classification must not be inherited by each descendant.

14.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Research Information Support (p. 160)	capability CT2	Canonical / Established
BC CT2: Health Innovation (p. 162)	capability CT2	Canonical / Established

14.7. BC CT2: Research Information Support

Property	Value	Meaning
Reference Id	`HCM-BC-research-information-support`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Research & Innovation	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:832 (p. 1348); business:834 (p. 1348); business:836 (p. 1348); business:844 (p. 1349)	Source standing does not approve the derived assertion.

14.7.1. Capability Overview

Research Information Support represents the ability to make existing health information available for a legitimate research purpose. Its single established child concentrates on governed provision, preserving the constraints under which information can be used rather than treating technical availability as research permission. The narrow decomposition reflects R1/R2 sufficiency rather than a claim to encompass every form of research support.

14.7.2. Definition / governed semantic detail

Provide the existing health information needed for authorised research within the deliberately bounded R1/R2 responsibility.

14.7.3. Intent

Support authorised research with information supplied under the applicable ethical, privacy and governance conditions.

14.7.4. Boundary

Research governance, trial oversight and specialist analysis remain unmodelled and are not absorbed into provision.

14.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Governed Research Information Provision (p. 161)	capability CT3	Canonical / Established

14.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Research Information Curation (p. 812)	Proposed

14.7.7. BC CT3: Governed Research Information Provision

Property	Value	Meaning
Reference Id	`HCM-BC-governed-research-information-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT.CT3-GOVERNED-RESEARCH-INFORMATION-PROVISION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Research & Innovation → BC CT2: Research Information Support	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:842 (p. 1349); business:838 (p. 1348); business:844 (p. 1349)	Source standing does not approve the derived assertion.

14.7.7.1. Capability Overview

Governed Research Information Provision makes existing health information available for an authorised research purpose under its applicable constraints. It represents a substantive information contribution while keeping ethics, governance, consent, privacy, security and permitted use material to provision. The responsibility concerns supplying existing information legitimately, rather than running the research that uses it.

14.7.7.2. Definition / governed semantic detail

Provide existing health information for an authorised research purpose under applicable ethics, governance, privacy, consent, security and information-use constraints.

14.7.7.3. Boundary

Availability is not research authorisation. Trials, genomic analysis and research-governance responsibilities are not subsumed by this deliberately narrow capability.

14.8. BC CT2: Health Innovation

Property	Value	Meaning
Reference Id	`HCM-BC-health-innovation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Research & Innovation	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:846 (p. 1349); business:848 (p. 1349); business:850 (p. 1349); business:858 (p. 1349)	Source standing does not approve the derived assertion.

14.8.1. Capability Overview

Health Innovation provides the business frame for considering changes in health-service and clinical practice. Its established evaluation child examines suitability, value and implications so that an innovation can be understood in the context in which it might be used. The model stops at that evaluative responsibility rather than presenting a complete pipeline from idea to operational adoption.

14.8.2. Definition / governed semantic detail

Consider health-service and clinical innovation within the R1/R2 business context.

14.8.3. Intent

Make consideration of innovation informed by its prospective benefits, consequences and suitability.

14.8.4. Boundary — v0.1 explanation

Evaluation does not establish an adoption decision, professional authority or an implementation programme.

14.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Innovation Evaluation (p. 163)	capability CT3	Canonical / Established

14.8.6. BC CT3: Health Innovation Evaluation

Property	Value	Meaning
Reference Id	`HCM-BC-health-innovation-evaluation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION.CT3-HEALTH-INNOVATION-EVALUATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Research & Innovation → BC CT2: Health Innovation	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:856 (p. 1349); business:852 (p. 1349); business:858 (p. 1349)	Source standing does not approve the derived assertion.

14.8.6.1. Capability Overview

Health Innovation Evaluation is the organisation's ability to form a considered view of a proposed or emerging innovation before treating it as suitable for use. Within Health Innovation it brings suitability, value and implications into the same evaluative context, so that potential benefit can be considered alongside consequences for the health-service or clinical setting. The outcome supports informed consideration of adoption rather than making adoption inevitable.

14.8.6.2. Definition / governed semantic detail

Evaluate proposed/emerging innovation for suitability, value, implications and potential adoption.

14.8.6.3. Intent

Give decision makers an intelligible basis for considering whether an innovation merits further adoption consideration.

14.8.6.4. Boundary — v0.1 explanation

Evaluation does not establish adoption, authorise a clinical change or create an implementation responsibility. The single established child does not imply a complete innovation lifecycle.

15. BC CT1: Health Service Direction & Stewardship

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-direction-stewardship`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:864 (p. 1349); business:868 (p. 1349); business:120 (p. 1332)	Source standing does not approve the derived assertion.

15.1. Capability Overview

This capability considers what health services should exist, what makes them viable and how they fit within the wider service ecosystem. Modelling frames the need and required abilities; alignment addresses dependencies, resources, financial arrangements and surrounding change. The distinction keeps the rationale for a service separate from its identifiable offering and its current operational delivery state.

15.2. Definition / governed semantic detail

Model needed health services and their viability requirements, support establishment/improvement and align services with the wider ecosystem of dependencies, resources, financial arrangements and organisational change.

15.3. Intent

Support viable, coherent service development and improvement within organisational and ecosystem constraints.

15.4. Boundary — v0.1 explanation

Identifying dependencies and resources does not acquire their operating authority. Service modelling and alignment do not establish the authoritative service catalogue.

15.5. Harmonia Architectural Relevance

Service need, viability and alignment facts materially shape service information and derivation requirements. That relevance does not assign executive authority or establish a future BEC mapping.

15.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Health Service Modelling (p. 166)	capability CT2	Canonical / Established
BC CT2: Health Service Alignment (p. 170)	capability CT2	Canonical / Established

15.7. BC CT2: Health Service Modelling

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-modelling`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:872 (p. 1349); business:874 (p. 1349); business:876 (p. 1349); business:887 (p. 1350)	Source standing does not approve the derived assertion.

15.7.1. Capability Overview

Health Service Modelling establishes the service needed and the ordinary business abilities required to make it viable. Its children distinguish recognising unmet need, identifying required abilities, supporting establishment and evaluating an existing service for improvement. The model can therefore revisit service assumptions without treating the original definition as permanently sufficient.

15.7.2. Definition / governed semantic detail

Establish what service should exist and what it needs to become viable.

15.7.3. Intent

Connect a service's purpose with the abilities and preparation required for it to exist usefully.

15.7.4. Boundary

Capability Identification uses ordinary business meaning; it does not allocate Harmonia capability tiers. Implementation support does not take over recruitment, facilities, procurement or ICT responsibilities.

15.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Service Need Identification (p. 167)	capability CT3	Canonical / Established
BC CT3: Health Service Capability Identification (p. 168)	capability CT3	Canonical / Established
BC CT3: Health Service Implementation Support (p. 169)	capability CT3	Canonical / Established
BC CT3: Health Service Continuous Improvement (p. 170)	capability CT3	Canonical / Established

15.7.6. BC CT3: Health Service Need Identification

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-need-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-NEED-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Modelling	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:882 (p. 1349); business:878 (p. 1349); business:887 (p. 1350)	Source standing does not approve the derived assertion.

15.7.6.1. Capability Overview

Health Service Need Identification establishes the unmet service need for which an organisational response is being considered. It gives service modelling a reason grounded in what is missing rather than beginning with a predetermined solution. That identified need can then

frame the abilities and resources required without assuming that a new offering has already been established.

15.7.6.2. Definition / governed semantic detail

Establish an unmet health-service need requiring response.

15.7.6.3. Boundary

Identifying need does not define an offering, approve funding or determine the surrounding implementation arrangements.

15.7.7. BC CT3: Health Service Capability Identification

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-capability-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-CAPABILITY-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Modelling	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:883 (p. 1349); business:878 (p. 1349); business:887 (p. 1350)	Source standing does not approve the derived assertion.

15.7.7.1. Capability Overview

This capability identifies the ordinary business abilities a proposed service needs across clinical, workforce, facility, information, technology and support concerns. It broadens service modelling beyond a clinical description to the conditions needed for viability. Its contribution is an understanding of required abilities, leaving their fulfilment and governing authority with the appropriate responsibilities.

15.7.7.2. Definition / governed semantic detail

Identify the ordinary business abilities needed to satisfy the service need across clinical, workforce, facility, technology, information and support concerns.

15.7.7.3. Boundary

The word capability here does not allocate Harmonia CT tiers or establish new catalogue elements.

15.7.8. BC CT3: Health Service Implementation Support

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-implementation-support`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-IMPLEMENTATION-SUPPORT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Modelling	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:884 (p. 1349); business:878 (p. 1349); business:887 (p. 1350)	Source standing does not approve the derived assertion.

15.7.8.1. Capability Overview

Health Service Implementation Support assists establishment of the framed service through organisational preparation and programmes of work. It connects the service model to practical readiness for introduction while recognising that recruitment, training, equipment and technology changes retain their own owners. Support therefore contributes to establishment without becoming an all-purpose implementation authority.

15.7.8.2. Definition / governed semantic detail

Support establishment of the framed service through applicable organisational preparation and programmes of work.

15.7.8.3. Boundary

It does not absorb HR, facilities, procurement or ICT responsibilities or establish operational-readiness ancestry.

15.7.9. BC CT3: Health Service Continuous Improvement

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-continuous-improvement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-CONTINUOUS-IMPROVEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Modelling	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:885 (p. 1349); business:878 (p. 1349); business:887 (p. 1350)	Source standing does not approve the derived assertion.

15.7.9.1. Capability Overview

This capability evaluates an established service and identifies correction or improvement needs across its service-wide purpose. It allows experience of actual operation to inform reconsideration of the service model and implementation support. The scope is broader than reporting a clinical-quality finding, while remaining anchored to the service being improved.

15.7.9.2. Definition / governed semantic detail

Evaluate the established service and identify correction/improvement needs that may feed back into modelling and implementation support.

15.7.9.3. Boundary — v0.1 explanation

It does not establish generic Continuous Improvement placement or transfer the authority of specialist activities affected by improvement.

15.8. BC CT2: Health Service Alignment

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-alignment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:889 (p. 1350); business:891 (p. 1350); business:893 (p. 1350); business:904 (p. 1350)	Source standing does not approve the derived assertion.

15.8.1. Capability Overview

Health Service Alignment positions a service among the other services, resources and arrangements on which it depends. It considers reciprocal dependencies, required resources, financial fit and the organisational changes needed for introduction or material change. Where modelling asks what service is needed, alignment asks how it can fit coherently into its environment.

15.8.2. Definition / governed semantic detail

Position the service coherently in its wider health-service ecosystem.

15.8.3. Intent

Expose the dependencies and surrounding commitments that affect service viability.

15.8.4. Boundary — v0.1 explanation

Alignment does not establish the service offering, own the required resources or settle the unresolved ancestry of collective operational readiness.

15.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Health Service Co-Dependency Management (p. 172)	capability CT3	Canonical / Established
BC CT3: Health Service Resource Identification (p. 172)	capability CT3	Canonical / Established
BC CT3: Health Service Financial Management (p. 173)	capability CT3	Canonical / Established
BC CT3: Health Service Implementation Planning (p. 174)	capability CT3	Canonical / Established

15.8.6. BC CT3: Health Service Co-Dependency Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-co-dependency-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-CO-DEPENDENCY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Alignment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:899 (p. 1350); business:895 (p. 1350); business:904 (p. 1350)	Source standing does not approve the derived assertion.

15.8.6.1. Capability Overview

Health Service Co-Dependency Management identifies and manages reciprocal needs between a service and its wider ecosystem. It makes explicit both what the service requires from others and what others rely on it to provide. This supports alignment by exposing dependencies that a service description considered in isolation may conceal.

15.8.6.2. Definition / governed semantic detail

Identify and manage what the service needs from others and what others need from it.

15.8.6.3. Boundary

Recognising a dependency does not acquire the provider’s responsibility or establish a new technical Service dependency.

15.8.7. BC CT3: Health Service Resource Identification

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-resource-identification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-RESOURCE-IDENTIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Alignment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:900 (p. 1350); business:895 (p. 1350); business:904 (p. 1350)	Source standing does not approve the derived assertion.

15.8.7.1. Capability Overview

This capability establishes the resources a service requires across workforce, facilities, equipment, clinical technology, ICT and support. It makes resource needs visible in the service's alignment context without treating identification as procurement, ownership or allocation. Required resources can therefore be discussed distinctly from resources presently available or deployed.

15.8.7.2. Definition / governed semantic detail

Identify required workforce, facilities, equipment, clinical technology, ICT, administration and support resources without owning them.

15.8.7.3. Boundary

Identifying resources does not operate them, assign them to care or establish collective service readiness.

15.8.8. BC CT3: Health Service Financial Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-financial-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-FINANCIAL-MANAGEMENT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Alignment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:901 (p. 1350); business:895 (p. 1350); business:904 (p. 1350)	Source standing does not approve the derived assertion.

15.8.8.1. Capability Overview

Health Service Financial Management establishes how the service fits applicable funding, financial and charging arrangements. It connects service viability with its surrounding financial context without attempting to become the enterprise finance function. The responsibility is service-specific alignment rather than the production of all financial transactions associated with care.

15.8.8.2. Definition / governed semantic detail

Establish the service's fit within funding, financial and charging arrangements without absorbing enterprise finance/accounting.

15.8.8.3. Boundary

Financial fit does not confer pricing, accounting, billing or enterprise revenue-management authority.

15.8.9. BC CT3: Health Service Implementation Planning

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-implementation-planning`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-IMPLEMENTATION-PLANNING`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Direction & Stewardship → BC CT2: Health Service Alignment	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:902 (p. 1350); business:895 (p. 1350); business:904 (p. 1350)	Source standing does not approve the derived assertion.

15.8.9.1. Capability Overview

Health Service Implementation Planning establishes and coordinates the organisational and surrounding service changes needed for introduction or material change. It considers how the service fits its environment, complementing modelling's establishment support with an alignment view of interdependent change. Its purpose is coherence of the planned transition rather than ownership of every change activity.

15.8.9.2. Definition / governed semantic detail

Establish and coordinate surrounding organisational/service changes needed to introduce or materially change the service.

15.8.9.3. Boundary

Planning does not perform all supporting changes or define the authoritative service offering.

16. BC CT1: Workforce & Organisational Capability

Property	Value	Meaning
Reference Id	`HCM-BC-workforce-organisational-capability`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:906 (p. 1350); business:910 (p. 1350); business:120 (p. 1332)	Source standing does not approve the derived assertion.

16.1. Capability Overview

This capability governs the workforce and organisational conditions under which health services can operate. Its workforce branch addresses professional standing, participation, privilege and enterprise coverage; its structural branch addresses organisations, recognised locations and identifiable service offerings. Directories can represent these facts, but their representation is separate from business authority to establish or change them.

16.2. Definition / governed semantic detail

Govern practitioner qualification, engagement/privilege and enterprise service coverage, together with organisation identity/structure, recognised locations and identifiable healthcare service offerings.

16.3. Intent

Make the organisational and workforce context of care explicit enough to constrain legitimate participation and service availability.

16.4. Boundary

Qualification, engagement, privilege, service coverage and deployment are distinct. Organisation or location association does not substitute for material Healthcare Service context.

16.5. Harmonia Architectural Relevance

These facts materially constrain care enablement, routing and operational context. Directory support preserves the originating workforce and organisational governance rather than making it platform-owned.

16.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Health Workforce Management (p. 177)	capability CT2	Canonical / Established
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	capability CT2	Canonical / Established

16.7. BC CT2: Health Workforce Management

Property	Value	Meaning
Reference Id	`HCM-BC-health-workforce-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:916 (p. 1350); business:918 (p. 1350); business:920 (p. 1350); business:930 (p. 1351)	Source standing does not approve the derived assertion.

16.7.1. Capability Overview

Health Workforce Management governs practitioners' professional standing, authorised participation and the ability to cover required enterprise work. Its three children separate qualification from engagement and privilege, and both from whole-enterprise coverage. This prevents an available or rostered person from being treated as automatically qualified and privileged for a particular activity.

16.7.2. Definition / governed semantic detail

Govern practitioners' professional standing, engagement/privilege and whole-enterprise service coverage.

16.7.3. Intent

Support legitimate practitioner participation and an informed understanding of service-coverage gaps and risks.

16.7.4. Boundary

Coverage is broader than rostering. Applying an available resource to a care requirement and progressing a particular support work item remain separate operational responsibilities.

16.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Practitioner Qualification Management (p. 178)	capability CT3	Canonical / Established
BC CT3: Practitioner Engagement & Privilege Management (p. 179)	capability CT3	Canonical / Established
BC CT3: Workforce Service Coverage Management (p. 180)	capability CT3	Canonical / Established

16.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Provider Administration (p. 243)	Proposed
To: enabled by	BEC CT2: Clinical Qualification Administration (p. 621)	Proposed
To: enabled by	BEC CT2: Clinical Rostering (p. 589)	Proposed

16.7.7. BC CT3: Practitioner Qualification Management

Property	Value	Meaning
Reference Id	`HCM-BC-practitioner-qualification-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-QUALIFICATION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability → BC CT2: Health Workforce Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	business:926 (p. 1350); business:922 (p. 1350); business:930 (p. 1351)	Source standing does not approve the derived assertion.

16.7.7.1. Capability Overview

Practitioner Qualification Management governs professional and clinical standing, including review and changes such as suspension or revocation. It makes qualification a continuing governed concern rather than a static directory attribute. That standing can constrain engagement and privilege while remaining distinguishable from either an employment relationship or permission for a particular activity.

16.7.7.2. Definition / governed semantic detail

Govern professional/clinical standing and qualifications, including applicable review, escalation, complaints, penalties/issues, suspension and revocation.

16.7.7.3. Boundary — v0.1 explanation

Qualification alone does not establish engagement, clinical privilege, device-specific enablement or operational assignment.

16.7.8. BC CT3: Practitioner Engagement & Privilege Management

Property	Value	Meaning
Reference Id	`HCM-BC-practitioner-engagement-privilege-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-ENGAGEMENT-PRIVILEGE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability → BC CT2: Health Workforce Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:927 (p. 1350); business:922 (p. 1350); business:930 (p. 1351)	Source standing does not approve the derived assertion.

16.7.8.1. Capability Overview

This capability manages how practitioners participate in services and the privileges applicable to that participation. It connects professional standing with roles and permitted capacities

in the service context, beyond ordinary employment administration. Separating engagement and privilege from qualification makes it possible to recognise a qualified practitioner who is not authorised for a particular service activity.

16.7.8.2. Definition / governed semantic detail

Manage practitioners as service participants, their roles/capacities, applicable privileges and qualification context above ordinary HR employment administration.

16.7.8.3. Boundary

Representation or consumption of privilege information does not transfer the underlying workforce decision authority to Harmonia.

16.7.9. BC CT3: Workforce Service Coverage Management

Property	Value	Meaning
Reference Id	`HCM-BC-workforce-service-coverage-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-WORKFORCE-SERVICE-COVERAGE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability → BC CT2: Health Workforce Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:928 (p. 1350); business:922 (p. 1350); business:930 (p. 1351)	Source standing does not approve the derived assertion.

16.7.9.1. Capability Overview

Workforce Service Coverage Management considers whether required enterprise work and service obligations can be fulfilled and how coverage should change when gaps or risks arise. Its concern is the adequacy of coverage, broader than the existence of a roster. This supplies a service-wide workforce view distinct from deploying a person to a particular care requirement.

16.7.9.2. Definition / governed semantic detail

Observe whether required enterprise work/service obligations can be fulfilled, identify coverage gaps/risks and adjust coverage in response.

16.7.9.3. Boundary

Coverage management is not individual support-work progression or automatic authority to assign clinical work.

16.8. BC CT2: Healthcare Organisation & Service Structure Management

Property	Value	Meaning
Reference Id	`HCM-BC-healthcare-organisation-service-structure-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:932 (p. 1351); business:934 (p. 1351); business:936 (p. 1351); business:946 (p. 1351)	Source standing does not approve the derived assertion.

16.8.1. Capability Overview

This capability establishes the organisation, location and service-offering context through which health services are recognised. Its children distinguish the identity and structure of participants, the definition of places, and what identifiable services are offered under temporal and conditional availability constraints. The distinction allows a service to exist without assuming it is offered everywhere or currently available.

16.8.2. Definition / governed semantic detail

Manage organisational identity/structure, recognised locations and identifiable organisational service offerings.

16.8.3. Intent

Provide an intelligible account of who offers what service, where and under which conditions.

16.8.4. Boundary — v0.1 explanation

Location definition is not facilities operation. Offering availability does not establish that all resources and dependencies are collectively ready to deliver the service.

16.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Healthcare Organisation Identity & Structure Management (p. 182)	capability CT3	Canonical / Established
BC CT3: Healthcare Location Definition Management (p. 183)	capability CT3	Canonical / Established
BC CT3: Healthcare Service Offering Management (p. 184)	capability CT3	Canonical / Established

16.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Organisation Administration (p. 263)	Proposed
To: enabled by	BEC CT2: Location Administration (p. 276)	Proposed
To: enabled by	BEC CT2: Health Service Administration (p. 287)	Proposed

16.8.7. BC CT3: Healthcare Organisation Identity & Structure Management

Property	Value	Meaning
Reference Id	`HCM-BC-healthcare-organisation-identity-structure-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-ORGANISATION-IDENTITY-STRUCTURE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability → BC CT2: Healthcare Organisation & Service Structure Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:942 (p. 1351); business:938 (p. 1351); business:946 (p. 1351)	Source standing does not approve the derived assertion.

16.8.7.1. Capability Overview

This capability establishes the identity and structural relationships of participating organisations and organisational units. It provides the business account of who the participants are and how they relate, which directory representations can then convey. The distinction matters because a represented hierarchy should not create organisational authority merely through document or interface nesting.

16.8.7.2. Definition / governed semantic detail

Establish and manage identity and structural relationships of participating organisations and organisational units.

16.8.7.3. Boundary

Organisation identity does not substitute for location, practitioner or material Healthcare Service context.

16.8.8. BC CT3: Healthcare Location Definition Management

Property	Value	Meaning
Reference Id	`HCM-BC-healthcare-location-definition-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-LOCATION-DEFINITION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability → BC CT2: Healthcare Organisation & Service Structure Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:943 (p. 1351); business:938 (p. 1351); business:946 (p. 1351)	Source standing does not approve the derived assertion.

16.8.8.1. Capability Overview

Healthcare Location Definition Management establishes recognised places and their business characteristics and relationships. It gives health-service activity an intelligible location context while remaining separate from maintaining the physical facilities. A defined location can be referenced in offerings and operational activity without implying that it is currently usable or ready for care.

16.8.8.2. Definition / governed semantic detail

Establish and manage recognised locations and their applicable business characteristics and relationships.

16.8.8.3. Boundary

Location definition is not facilities operation, care-place readiness or service availability.

16.8.9. BC CT3: Healthcare Service Offering Management

Property	Value	Meaning
Reference Id	`HCM-BC-healthcare-service-offering-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-SERVICE-OFFERING-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Workforce & Organisational Capability → BC CT2: Healthcare Organisation & Service Structure Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:944 (p. 1351); business:938 (p. 1351); business:946 (p. 1351)	Source standing does not approve the derived assertion.

16.8.9.1. Capability Overview

This capability establishes the identifiable service offered, including provider, place, timing and availability conditions. It makes a service discoverable as an offering rather than merely an abstract service idea. Temporal and conditional availability are material: an offering may exist but be unavailable in the present context, and another pathway may cover different hours.

16.8.9.2. Definition / governed semantic detail

Establish/manage what identifiable service is offered, by whom, where, when and under which availability conditions/constraints.

16.8.9.3. Boundary — v0.1 explanation

An available offering does not establish collective readiness to deliver it; organisation and location alone do not supply the service context.

17. BC CT1: Corporate Resources & Enterprise Services

Property	Value	Meaning
Reference Id	`HCM-BC-corporate-resources-enterprise-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES`	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:948 (p. 1351); business:952 (p. 1351); business:120 (p. 1332)	Source standing does not approve the derived assertion.

17.1. Capability Overview

This capability retains the financial and corporate-support context material to R1/R2. The financial branch concentrates on information about clinical activity relevant to charging or cost; corporate support covers administrative, physical, technical, people and communication conditions affecting service activity. The limited decomposition makes these dependencies visible without assigning a complete corporate enterprise architecture to Harmonia.

17.2. Definition / governed semantic detail

Provide the bounded clinical-activity charging information and enterprise support context required for R1/R2, without assigning enterprise finance, HR, facilities or ICT operating authority to Harmonia.

17.3. Intent

Support health-service activity with the required corporate context and legitimate activity information.

17.4. Boundary

Evidence about cost-relevant activity does not perform billing or accounting. Consuming corporate resource state does not acquire the authority to operate those resources.

17.5. Harmonia Architectural Relevance

Clinical activity evidence provides material financial support. Corporate constraints may also affect enablement, but contextual consumption does not classify every corporate descendant as Relevant or Core.

17.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Financial & Commercial Services (p. 186)	capability CT2	Canonical / Established
BC CT2: Corporate Support Services (p. 188)	capability CT2	Canonical / Established

17.7. BC CT2: Financial & Commercial Services

Property	Value	Meaning
Reference Id	`HCM-BC-financial-commercial-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:956 (p. 1351); business:958 (p. 1351); business:960 (p. 1351); business:968 (p. 1352)	Source standing does not approve the derived assertion.

17.7.1. Capability Overview

Financial & Commercial Services provides the financial business frame needed for the bounded R1/R2 responsibility. Its established child identifies and supplies information about chargeable or cost-relevant clinical activity. The purpose is to make applicable activity evidence available to the responsible financial context without expanding the model into pricing, claims or revenue management.

17.7.2. Definition / governed semantic detail

Provide the financial/commercial business context materially required for R1/R2.

17.7.3. Intent

Support legitimate funding, financial and charging purposes with relevant clinical-activity information.

17.7.4. Boundary — v0.1 explanation

Not every exchanged request or activity is chargeable. Provision of evidence does not establish a charging decision or an enterprise financial system.

17.7.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Clinical Activity Charging Information Provision (p. 187)	capability CT3	Canonical / Established

17.7.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT2: Funding & Coverage Administration (p. 487)	Proposed
To: enabled by	BEC CT2: Clinical Charging & Claims Administration (p. 489)	Proposed

17.7.7. BC CT3: Clinical Activity Charging Information Provision

Property	Value	Meaning
Reference Id	`HCM-BC-clinical-activity-charging-information-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES.CT3-CLINICAL-ACTIVITY-CHARGING-INFORMATION-PROVISION`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Financial & Commercial Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:966 (p. 1352); business:962 (p. 1351); business:968 (p. 1352)	Source standing does not approve the derived assertion.

17.7.7.1. Capability Overview

This capability identifies and supplies information about clinical or health-service activity relevant to legitimate charging, funding or cost purposes. Requests, service types, quantities and utilisation context may contribute evidence, but chargeability cannot be inferred from the fact that Harmonia mediated an interaction. Its outcome is usable activity information for the responsible financial context.

17.7.7.2. Definition / governed semantic detail

Identify and provide information about chargeable or cost-relevant clinical/health-service activity where required for applicable funding, financial or charging purposes.

17.7.7.3. Boundary

Evidence provision does not perform pricing, billing, claims, accounting or revenue management.

17.8. BC CT2: Corporate Support Services

Property	Value	Meaning
Reference Id	`HCM-BC-corporate-support-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:970 (p. 1352); business:972 (p. 1352); business:974 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.1. Capability Overview

Corporate Support Services frames enterprise support whose resources, availability or constraints affect health-service activity. Its children retain recognisable administrative, facilities, infrastructure, communications, people and operational-oversight responsibilities. This explains why their context matters to service delivery while keeping the people who operate those concerns accountable for them.

17.8.2. Definition / governed semantic detail

Provide enterprise support whose resources, availability, state or constraints may materially affect service activity.

17.8.3. Intent

Make material support conditions visible to health-service activity without confusing consumption with ownership.

17.8.4. Boundary

No further R1/R2 decomposition is established. Context from these abilities does not settle the collective service-readiness placement question.

17.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Back-Office Services (p. 189)	capability CT3	Canonical / Established
BC CT3: Facilities Management (p. 190)	capability CT3	Canonical / Established
BC CT3: IT Infrastructure Management (p. 191)	capability CT3	Canonical / Established
BC CT3: Communications & Publicity (p. 192)	capability CT3	Canonical / Established
BC CT3: People Management (p. 192)	capability CT3	Canonical / Established
BC CT3: Operational Oversight (p. 193)	capability CT3	Canonical / Established

17.8.6. BC CT3: Back-Office Services

Property	Value	Meaning
Reference Id	`HCM-BC-back-office-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-BACK-OFFICE-SERVICES`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Corporate Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:980 (p. 1352); business:976 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.6.1. Capability Overview

Back-Office Services provides the administrative and support activity on which health-service operations depend. Within Corporate Support Services it retains an enterprise context for dependencies that may affect whether service activity can proceed. The model stops at this broad responsibility rather than allocating each administrative function to Harmonia.

17.8.6.2. Definition / governed semantic detail

Provide administrative/support services on which health-service activity depends.

17.8.6.3. Boundary

Consumption of support status does not transfer administrative operating authority or establish a more detailed decomposition.

17.8.7. BC CT3: Facilities Management

Property	Value	Meaning
Reference Id	`HCM-BC-facilities-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-FACILITIES-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Corporate Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:981 (p. 1352); business:976 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.7.1. Capability Overview

Facilities Management manages physical facilities and services, including the state and availability context relevant to health activity. It contributes the real-world conditions that location definitions alone cannot establish. This distinguishes a place recognised in a directory from a facility being operated and maintained for use.

17.8.7.2. Definition / governed semantic detail

Manage physical facilities/services and applicable resource, location, availability and facility-state context.

17.8.7.3. Boundary

Harmonia's consumption of facility constraints does not make it the facilities operator or establish collective service readiness.

17.8.8. BC CT3: IT Infrastructure Management

Property	Value	Meaning
Reference Id	`HCM-BC-it-infrastructure-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-IT-INFRASTRUCTURE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Corporate Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:982 (p. 1352); business:976 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.8.1. Capability Overview

IT Infrastructure Management operates the underlying technology infrastructure and its availability. It supplies an enterprise support context for health-service activity that relies on functioning technology. Its business outcome is operational infrastructure, distinct from governance of health-information meaning or the application responsibilities realised on that infrastructure.

17.8.8.2. Definition / governed semantic detail

Operate/manage underlying technology infrastructure and its availability.

17.8.8.3. Boundary

Infrastructure machinery does not define Harmonia's information semantics or acquire clinical authority.

17.8.9. BC CT3: Communications & Publicity

Property	Value	Meaning
Reference Id	`HCM-BC-communications-publicity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-COMMUNICATIONS-PUBLICITY`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Corporate Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:983 (p. 1352); business:976 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.9.1. Capability Overview

Communications & Publicity manages organisational publication and communication arrangements relevant to information availability and distribution. It provides enterprise communication context without being identical to clinical record exchange or digitally mediated participant interaction. The governed decomposition retains that broad support role without adding specific publicity or communications functions.

17.8.9.2. Definition / governed semantic detail

Manage organisational communication/publication arrangements relevant to information availability/distribution.

17.8.9.3. Boundary

Organisational communication does not originate the clinical facts or governed conclusions it may convey.

17.8.10. BC CT3: People Management

Property	Value	Meaning
Reference Id	`HCM-BC-people-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-PEOPLE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Corporate Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:984 (p. 1352); business:976 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.10.1. Capability Overview

People Management addresses people across the organisation as an enterprise support responsibility. It retains a scope broader than the practitioner-specific standing, privilege and coverage concerns of Health Workforce Management. This distinction allows health-service activity to consume relevant people context without redefining all workforce administration as clinical governance.

17.8.10.2. Definition / governed semantic detail

Manage people across the organisation, distinct from practitioner-specific Health Workforce Management.

17.8.10.3. Boundary

General people administration does not itself establish practitioner clinical qualification or privilege.

17.8.11. BC CT3: Operational Oversight

Property	Value	Meaning
Reference Id	`HCM-BC-operational-oversight`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-OPERATIONAL-OVERSIGHT`	Current proposed context where marked Proposed.
Current context	BC CT1: Corporate Resources & Enterprise Services → BC CT2: Corporate Support Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:985 (p. 1352); business:976 (p. 1352); business:987 (p. 1352)	Source standing does not approve the derived assertion.

17.8.11.1. Capability Overview

Operational Oversight observes broader operating conditions affecting the organisation's ability to work and deliver services. It provides context beyond an individual work item or care place, helping make material constraints visible. Within corporate support it remains a broad enterprise responsibility rather than an inferred owner for every operational-readiness question.

17.8.11.2. Definition / governed semantic detail

Observe broader operational conditions affecting the organisation's ability to work or deliver services.

17.8.11.3. Boundary

Oversight does not resolve collective service-readiness placement or constitute independent assurance merely through observation.

18. BC CT1: Health Service Governance

Property	Value	Meaning
Reference Id	HCM-BC-health-service-governance	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-HEALTH-SERVICE-GOVERNANCE	Current proposed context where marked Proposed.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:993 (p. 1352); business:997 (p. 1352); business:120 (p. 1332)	Source standing does not approve the derived assertion.

18.1. Capability Overview

Health Service Governance establishes the normative envelope within which health-service activity operates: what obligations apply, who may decide, what constraints bind decisions and how adherence will be assessed and reported. Its framework and compliance branches distinguish the authoritative requirements from the arrangements used to judge satisfaction. Governance therefore shapes legitimate activity without becoming the manager of every governed operation.

18.2. Definition / governed semantic detail

Establish and maintain the obligations, authorities, constraints, policies and governance framework under which health-service activity must operate, and define how adherence will be assessed, checked and reported.

18.3. Intent

Make obligations, decision authority and compliance expectations explicit and traceable.

18.4. Boundary

Operational management chooses and progresses means within the governing envelope; assurance evaluates evidence. Neither responsibility is absorbed merely because governance defines its constraints.

18.5. Harmonia Architectural Relevance — v0.1 explanation

Formal relevance remains Unresolved. The requirements plainly constrain managed activity, but the RAD has not assigned enough platform participation or enabling derivation to adjudicate its relevance classification.

18.6. Preserved v0.1 Review Matters

No Business Enabling derivation is established for Health Service Governance.

18.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Governance Framework Management (p. 196)	capability CT2	Canonical / Established
BC CT2: Compliance Governance (p. 199)	capability CT2	Canonical / Established

18.8. BC CT2: Governance Framework Management

Property	Value	Meaning
Reference Id	`HCM-BC-governance-framework-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1003 (p. 1352); business:1005 (p. 1353); business:1007 (p. 1353); business:1016 (p. 1353)	Source standing does not approve the derived assertion.

18.8.1. Capability Overview

Governance Framework Management establishes what must be satisfied and the legitimate authority and bounds for organisational decisions. Its two children separate obligations attributable to source authority from the allocation and limitation of decision authority. Together they supply the authoritative reference against which operational arrangements and compliance assessments can be considered.

18.8.2. Definition / governed semantic detail

Establish the authoritative WHAT and legitimate working parameters.

18.8.3. Intent

Keep required outcomes and legitimate decision parameters distinguishable and traceable to their authority.

18.8.4. Boundary

Interpreting an external obligation or engaging with a regulator does not transfer that body's authority. Operational means remain with management.

18.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Policy / Obligation Governance (p. 197)	capability CT3	Canonical / Established
BC CT3: Authority & Constraint Governance (p. 198)	capability CT3	Canonical / Established

18.8.6. BC CT3: Policy / Obligation Governance

Property	Value	Meaning
Reference Id	`HCM-BC-policy-obligation-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-POLICY-OBLIGATION-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Governance → BC CT2: Governance Framework Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1013 (p. 1353); business:1009 (p. 1353); business:1016 (p. 1353)	Source standing does not approve the derived assertion.

18.8.6.1. Capability Overview

Policy / Obligation Governance establishes and maintains the authoritative requirements under which the organisation operates. It interprets applicable legislative, regulatory, contractual,

policy and other sources while preserving traceability to their authority. The purpose is a legitimate account of what must be satisfied, rather than an operational choice about how to satisfy it.

18.8.6.2. Definition / governed semantic detail

Identify, interpret, establish and maintain authoritative legislative, regulatory, policy, standards, guideline, contractual and other obligations, attributable and traceable to source authority.

18.8.6.3. Boundary

Interpreting obligations does not acquire the regulator's authority or determine operational means.

18.8.7. BC CT3: Authority & Constraint Governance

Property	Value	Meaning
Reference Id	`HCM-BC-authority-constraint-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-AUTHORITY-CONSTRAINT-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Governance → BC CT2: Governance Framework Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1014 (p. 1353); business:1009 (p. 1353); business:1016 (p. 1353)	Source standing does not approve the derived assertion.

18.8.7.1. Capability Overview

This capability establishes who may make or approve organisational decisions and the bounds within which that authority applies. It complements obligation governance by distinguishing required outcomes from legitimate decision powers. A participant can therefore be responsible for acting within constraints without being authorised to redefine the governing requirement.

18.8.7.2. Definition / governed semantic detail

Establish who may make, approve or govern organisational decisions/artefacts and the bounds within which authority may be exercised.

18.8.7.3. Boundary

Authority allocation does not itself perform an operational decision or a runtime access-control evaluation.

18.9. BC CT2: Compliance Governance

Property	Value	Meaning
Reference Id	`HCM-BC-compliance-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1018 (p. 1353); business:1020 (p. 1353); business:1022 (p. 1353); business:1031 (p. 1353)	Source standing does not approve the derived assertion.

18.9.1. Capability Overview

Compliance Governance defines how adherence to established obligations, authorities and constraints will be determined. Assessment Definition specifies the evidence and basis for evaluation; Reporting & Checkpoint Definition specifies where adherence is checked and what must be communicated. This makes the compliance test explicit without treating the design of a test as its successful execution.

18.9.2. Definition / governed semantic detail

Define how adherence to the authoritative WHAT will be determined, checked and reported.

18.9.3. Intent

Provide a defensible basis for checking and reporting whether the governing envelope has been respected.

18.9.4. Boundary

Defining assessment arrangements does not perform assurance or manage the governed activity. The obligations being assessed retain their authoritative source.

18.9.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Compliance Assessment Definition (p. 200)	capability CT3	Canonical / Established
BC CT3: Governance Reporting & Checkpoint Definition (p. 201)	capability CT3	Canonical / Established

18.9.6. BC CT3: Compliance Assessment Definition

Property	Value	Meaning
Reference Id	`HCM-BC-compliance-assessment-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-COMPLIANCE-ASSESSMENT-DEFINITION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Governance → BC CT2: Compliance Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1028 (p. 1353); business:1024 (p. 1353); business:1031 (p. 1353)	Source standing does not approve the derived assertion.

18.9.6.1. Capability Overview

Compliance Assessment Definition specifies the evidence, measures, criteria and arrangements sufficient to evaluate adherence. It makes the assessment basis explicit after obligations, authorities and constraints have been established. This preserves the distinction between defining what compliance means and determining whether the available evidence demonstrates it in a particular case.

18.9.6.2. Definition / governed semantic detail

Define the evidence, measures, criteria, controls and assessment arrangements sufficient to evaluate compliance with established obligations, authorities and constraints.

18.9.6.3. Boundary

Assessment definition does not replace the authoritative obligation, perform assurance or establish a compliance finding.

18.9.7. BC CT3: Governance Reporting & Checkpoint Definition

Property	Value	Meaning
Reference Id	`HCM-BC-governance-reporting-checkpoint-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-GOVERNANCE-REPORTING-CHECKPOINT-DEFINITION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Governance → BC CT2: Compliance Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1029 (p. 1353); business:1024 (p. 1353); business:1031 (p. 1353)	Source standing does not approve the derived assertion.

18.9.7.1. Capability Overview

This capability specifies where adherence must be checked and what governance or compliance information must be reported. It gives the governance framework observable expectations without treating reporting requirements as completed reporting. Within Compliance Governance it complements the assessment basis by making checking and communication obligations explicit.

18.9.7.2. Definition / governed semantic detail

Establish where/how adherence is checked and what governance/compliance information must be reported.

18.9.7.3. Boundary

Defining a checkpoint does not perform the check or take over progression of the governed operation.

19. BC CT1: Health Service Assurance

Property	Value	Meaning
Reference Id	HCM-BC-health-service-assurance	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Established	Root placement.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-HEALTH-SERVICE-ASSURANCE	Current proposed context where marked Proposed.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1033 (p. 1353); business:1037 (p. 1353); business:120 (p. 1332)	Source standing does not approve the derived assertion.

19.1. Capability Overview

Health Service Assurance establishes justified confidence in the ability to deliver health services appropriately. Specification defines how confidence will be established; evaluation determines what the available evidence supports. Its subject is macro-level service delivery, with workforce, clinical, information, security and other concerns contributing evidence or criteria without becoming separate assurance trees.

Where independent governed assurance is required, the subject cannot control its own evaluation or conclusion. Independence concerns control of the assurance judgement rather than requiring a separate organisation or technology installation.

19.2. Definition / governed semantic detail

Establish and maintain justified confidence that health services can be, and are being, delivered in accordance with applicable expectations, obligations, constraints and required levels of performance.

19.3. Intent

Distinguish justified confidence, qualified conclusions and insufficient evidence so that assurance communicates what is actually supported.

19.4. Boundary

Assurance manages its own evaluation and does not assign, remediate or progress the subject activity. Harmonia does not thereby assume responsibility for clinical adequacy or professional

practice.

19.5. Harmonia Architectural Relevance

Approved independent governed assurance materially supports service confidence. The three established assurance BECs provide bounded support; they are not remapped to the newer BC descendants and are not assigned implementation components.

19.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT2: Health Service Assurance Specification (p. 203)	capability CT2	Canonical / Established
BC CT2: Assurance Evaluation (p. 206)	capability CT2	Canonical / Established

19.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enabled by	BEC CT3: Assurance Design (p. 824)	Established
To: enabled by	BEC CT3: Assurance Criteria Administration (p. 828)	Established

19.8. BC CT2: Health Service Assurance Specification

Property	Value	Meaning
Reference Id	`HCM-BC-health-service-assurance-specification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Assurance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1043 (p. 1353); business:1045 (p. 1354); business:1047 (p. 1354); business:1056 (p. 1354)	Source standing does not approve the derived assertion.

19.8.1. Capability Overview

This capability defines the approach and evaluation criteria through which service-delivery assurance can be justified. Approach Definition addresses scope, method, responsibility, evidence

and applicable independence; Criteria Governance establishes the basis for judging sufficiency. Their separation distinguishes the design of an assurance activity from the normative or evaluative material it applies.

19.8.2. Definition / governed semantic detail

Specify how macro-level service-delivery assurance will be undertaken and what sufficiently supports confidence.

19.8.3. Intent

Make the intended basis of confidence explicit before a conclusion is treated as assurance.

19.8.4. Boundary

An approach cannot waive a higher-level independence requirement. Contributing specialist concerns do not create separate Business Assurance hierarchies.

19.8.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Assurance Approach Definition (p. 204)	capability CT3	Canonical / Established
BC CT3: Assurance Evaluation Criteria Governance (p. 205)	capability CT3	Canonical / Established

19.8.6. BC CT3: Assurance Approach Definition

Property	Value	Meaning
Reference Id	`HCM-BC-assurance-approach-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-APPROACH-DEFINITION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Assurance → BC CT2: Health Service Assurance Specification	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1053 (p. 1354); business:1049 (p. 1354); business:1056 (p. 1354)	Source standing does not approve the derived assertion.

19.8.6.1. Capability Overview

Assurance Approach Definition establishes how service-delivery assurance will be undertaken, including scope, evidence, methods, responsibilities, timing and required independence. It gives assurance activity a defensible approach rather than assuming any available observation can support confidence. The approach must respect applicable higher-level requirements when deciding what constitutes sufficient evaluation.

19.8.6.2. Definition / governed semantic detail

Define scope, evidence expectations, responsibilities, methods, applicable degree of independence and timing/frequency for service-delivery assurance.

19.8.6.3. Boundary

Self-attestation cannot replace required independent governed assurance, and an approach does not itself establish a conclusion.

19.8.7. BC CT3: Assurance Evaluation Criteria Governance

Property	Value	Meaning
Reference Id	`HCM-BC-assurance-evaluation-criteria-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-EVALUATION-CRITERIA-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Assurance → BC CT2: Health Service Assurance Specification	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1054 (p. 1354); business:1049 (p. 1354); business:1056 (p. 1354)	Source standing does not approve the derived assertion.

19.8.7.1. Capability Overview

This capability establishes and maintains the criteria against which assurance sufficiency is judged. It makes the evaluative basis of confidence explicit across the different concerns that can affect service delivery. Criteria can draw on clinical, workforce, technology, information or other contexts without turning those subjects into independent assurance capability trees.

19.8.7.2. Definition / governed semantic detail

Establish and maintain criteria against which sufficient assurance can be determined.

19.8.7.3. Boundary

Criteria governance is distinct from applying criteria to evidence and does not remap the existing assurance BEC responsibilities.

19.9. BC CT2: Assurance Evaluation

Property	Value	Meaning
Reference Id	`HCM-BC-assurance-evaluation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Assurance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1058 (p. 1354); business:1060 (p. 1354); business:1062 (p. 1354); business:1071 (p. 1354)	Source standing does not approve the derived assertion.

19.9.1. Capability Overview

Assurance Evaluation determines what confidence the evidence legitimately supports under the established approach and criteria. Evidence Assessment considers the material against that basis; Conclusion Determination establishes the resulting position, including qualifications and unresolved concerns. The distinction prevents accumulated evidence or execution completion from being mistaken for a justified assurance conclusion.

19.9.2. Definition / governed semantic detail

Determine what justified confidence the applicable evidence supports.

19.9.3. Intent

State the supported level and limits of confidence without concealing insufficient evidence.

19.9.4. Boundary

Insufficient evidence establishes neither assurance nor non-assurance. Operational response remains with the activity's responsible capability, and the existing assurance BEC ownership is unchanged.

19.9.5. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BC CT3: Assurance Evidence Assessment (p. 207)	capability CT3	Canonical / Established
BC CT3: Assurance Conclusion Determination (p. 208)	capability CT3	Canonical / Established

19.9.6. BC CT3: Assurance Evidence Assessment

Property	Value	Meaning
Reference Id	`HCM-BC-assurance-evidence-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-EVIDENCE-ASSESSMENT`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Assurance → BC CT2: Assurance Evaluation	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1068 (p. 1354); business:1064 (p. 1354); business:1071 (p. 1354)	Source standing does not approve the derived assertion.

19.9.6.1. Capability Overview

Assurance Evidence Assessment evaluates available evidence against the established approach and criteria. It produces the assessed basis from which a legitimate conclusion may be determined, retaining qualifications and evidentiary limits. The responsibility is evaluative rather than the operational production of the source information or remediation of what it reveals.

19.9.6.2. Definition / governed semantic detail

Evaluate available evidence against the established assurance approach and criteria.

19.9.6.3. Boundary

Assessment is distinct from conclusion determination; unavailable or insufficient evidence must remain explicit.

19.9.7. BC CT3: Assurance Conclusion Determination

Property	Value	Meaning
Reference Id	`HCM-BC-assurance-conclusion-determination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Established	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-CONCLUSION-DETERMINATION`	Current proposed context where marked Proposed.
Current context	BC CT1: Health Service Assurance → BC CT2: Assurance Evaluation	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	business:1069 (p. 1354); business:1064 (p. 1354); business:1071 (p. 1354)	Source standing does not approve the derived assertion.

19.9.7.1. Capability Overview

Assurance Conclusion Determination establishes what confidence is legitimately supported by the assessment. It preserves qualifications and unresolved concerns rather than forcing incomplete evidence into a positive or negative result. Within Assurance Evaluation it is the responsibility that gives the conclusion its standing, distinct from gathering or assessing the supporting material.

19.9.7.2. Definition / governed semantic detail

Establish the legitimately supported conclusion, including insufficiency, qualification and unresolved concerns where applicable.

19.9.7.3. Boundary

Insufficiency proves neither assurance nor non-assurance, and the conclusion does not manage the subject's operational response.

Part III.

Business Enabling Capability Architecture

The seven CT1 branches follow the human-reviewed candidate hierarchy. Compatible existing responsibilities and meaningful Feature frames are retained or relocated; structural relationships remain Proposed. Explicit Not Addressed in R1.x / R2.x annotations are scope statements, independent of architectural status. Superseded records and unresolved semantic conflicts are preserved in a separate register, outside this current hierarchy.

CT1 / page	Element status	Placement
BEC CT1: Entity Information Enablement (p. 211)	Proposed	Proposed
BEC CT1: Care Administration Enablement (p. 326)	Proposed	Proposed
BEC CT1: Clinical Service Dependency Management (p. 491)	Proposed	Proposed
BEC CT1: Health Service Operations Enablement (p. 512)	Proposed	Proposed
BEC CT1: Shared Information & Coordination Enablement (p. 654)	Proposed	Proposed
BEC CT1: Health Informatics Management (p. 811)	Proposed	Proposed
BEC CT1: Independent Assurance Enablement (p. 821)	Proposed	Proposed

20. BEC CT1: Entity Information Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-healthcare-entity-information-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT`	Current proposed context where marked Proposed.
Name history	Healthcare Entity Information Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668	Source standing does not approve the derived assertion.

20.1. Capability Overview

Administer identifying, descriptive and qualified relationship information concerning healthcare participants, organisations, locations, services, materiel and devices.

20.2. Definition — Reviewed candidate

Administer identifying, descriptive and qualified relationship information concerning healthcare participants, organisations, locations, services, materiel and devices.

20.3. Boundary

Registry information does not establish clinical privilege, service readiness, occupancy or originating national identity authority.

20.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668

20.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Maintain the identity, descriptive structure and qualified relationships of the participants, places, services, products and devices referenced in healthcare activity.

Previous overview: A care transaction needs to identify its subject, provider, service and place before it can be interpreted safely. This frame brings together those relatively enduring descriptions while keeping each kind of entity and relationship accountable within its own boundary.

Previous intent: Make the participant, place and service references in a care concern consistently interpretable across organisational boundaries.

20.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Client Administration (p. 212)	capability CT2	Canonical / Proposed
BEC CT2: Provider Administration (p. 243)	capability CT2	Canonical / Proposed
BEC CT2: Organisation Administration (p. 263)	capability CT2	Canonical / Proposed
BEC CT2: Location Administration (p. 276)	capability CT2	Canonical / Proposed
BEC CT2: Health Service Administration (p. 287)	capability CT2	Canonical / Proposed
BEC CT2: Health Materiel Administration (p. 300)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Device Administration (p. 305)	capability CT2	Canonical / Proposed

20.9. BEC CT2: Client Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-client-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:76 (p. 1365); enabling:112 (p. 1366)	Source standing does not approve the derived assertion.

20.9.1. Capability Overview

Client Administration provides the representation and demographic context needed to recognise individuals receiving care. It brings identity, healthcare-subject context, relationships and privacy into the same administrative frame while preserving their different responsibilities. The established partial chain to Person Identity is retained; the catalogue's other contextual descriptions do not supply a complete capability hierarchy.

20.9.2. Definition — Reviewed candidate

Client Administration provides the representation and demographic context needed to recognise individuals receiving care. It brings identity, healthcare-subject context, relationships and privacy into the same administrative frame while preserving their different responsibilities. The established partial chain to Person Identity is retained; the catalogue's other contextual descriptions do not supply a complete capability hierarchy.

20.9.3. Intent — retained v0.1 source explanation

Keep the individual and the circumstances of their representation intelligible across care information and operational activity.

20.9.4. Boundary — retained v0.1 source explanation

R1/R2 identifier correlation and administrative correction do not make Harmonia the authoritative clinical EMPI. Probabilistic matching and master identity merge/unmerge remain an uncommitted roadmap candidate.

20.9.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.9.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.9.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.9.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Person Identity Administration (p. 214)	capability CT3	Canonical / Proposed
BEC CT3: Healthcare Subject Profile Administration (p. 226)	capability CT3	Canonical / Proposed
BEC CT3: Client Relationship Administration (p. 232)	capability CT3	Canonical / Proposed
BEC CT3: Client Privacy Guardrail Management (p. 237)	capability CT3	Canonical / Proposed

20.9.9. BEC CT3: Person Identity Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-person-identity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration	Linked responsibility references; historical records do not assert current ancestry.
Established name	Person Identity	Current display rename is Proposed; meaning unchanged.
Name history	Person Identity	Names do not establish another identity.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:115 (p. 1366); em:16 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.1. Capability Overview

Resolve, maintain and correlate identities and identifiers assigned to a person across issuing authorities, retaining source-qualified correction history.

20.9.9.2. Preserved Canonical semantic evidence — source context

Resolving, maintaining, and correlating identities and identifiers assigned to an individual across different authorities.

20.9.9.3. Definition — Reviewed candidate

Resolve, maintain and correlate identities and identifiers assigned to a person across issuing authorities, retaining source-qualified correction history.

20.9.9.4. Intent — retained v0.1 source explanation

Support reliable association of care information with the intended individual across participating systems.

20.9.9.5. Boundary — retained v0.1 source explanation

Harmonia relies on authoritative upstream PAS/MPI information for R1/R2. Correlation and correction notifications do not establish an autonomous master-patient reconciliation engine.

20.9.9.6. Harmonia Architectural Relevance — retained v0.1 source explanation

Identity association is a direct platform concern because clinical records, orders and operational activity need an enduring subject context. Its Core standing does not imply originating master identity authority.

20.9.9.7. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.9.9.8. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.9.9.9. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.9.9.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Identifier Resolution (p. 216)	feature FT	Canonical / Proposed
Feature: Cross-Authority Identifier Correlation (p. 217)	feature FT	Canonical / Proposed
Feature: Identity Alias Association (p. 218)	feature FT	Canonical / Proposed
Feature: Governed Identity Correction (p. 218)	feature FT	Canonical / Proposed

Element / page	Type / tier	Element / placement status
Function: Resolve Person Identifier (p. 219)	function	Canonical / Established
Service: Person Identifier Resolution (p. 220)	service	Canonical / Established
Function: Correlate Person Identifiers (p. 220)	function	Canonical / Established
Service: Person Identifier Correlation (p. 221)	service	Canonical / Established
Function: Maintain Person Identity Aliases (p. 222)	function	Canonical / Established
Function: Apply Governed Person Identity Correction (p. 222)	function	Canonical / Established
Service: Person Identity Correction (p. 223)	service	Canonical / Established
Process: Governed Person Identity Correction Process (p. 224)	process	Canonical / Established
Feature: Identity Source & Correction Lineage (p. 225)	feature FT	Proposed / Proposed

20.9.9.11. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed
To: enterprise enabled by	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed

20.9.9.12. Feature: Identifier Resolution

Property	Value	Meaning
Reference Id	`FEAT-EM-01`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-01`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:117 (p. 1366)	Source standing does not approve the derived assertion.

20.9.9.12.1. Feature Overview

Identifier Resolution interprets an identifier in its issuing-authority and namespace context, so that the same local value is not assumed to identify the same person everywhere. It provides an identifiable subject reference without deciding an authoritative master-person match.

20.9.9.12.2. Definition / governed semantic detail

Ingest and resolve external identifiers against registered issuing authorities and namespace systems.

20.9.9.12.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Resolve Person Identifier (p. 219)	Established
From: associated feature	Service: Person Identifier Resolution (p. 220)	Established

20.9.9.13. Feature: Cross-Authority Identifier Correlation

Property	Value	Meaning
Reference Id	`FEAT-EM-02`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-02`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:118 (p. 1366)	Source standing does not approve the derived assertion.

20.9.9.13.1. Feature Overview

Cross-authority correlation preserves the association between identifiers used for one person by different participants. It enables information to be related across regional systems without erasing the separate identifier namespaces or claiming that Harmonia originated the identity decision.

20.9.9.13.2. Definition / governed semantic detail

Correlate disparate local identifiers (e.g., hospital MRN, clinic client ID, national IHI) belonging to the same individual.

20.9.9.13.3. Prior Candidate Derivation

v0.3 reconciliation: `BEC-D5D59967` (line 2686); `BEC-C7BBD105` (line 3050); `BEC-314BE2D1` (line 3151). Working locators are not identity aliases.

20.9.9.13.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Correlate Person Identifiers (p. 220)	Established
From: associated feature	Service: Person Identifier Correlation (p. 221)	Established

20.9.9.14. Feature: Identity Alias Association

Property	Value	Meaning
Reference Id	`FEAT-EM-03`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-03`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:119 (p. 1366)	Source standing does not approve the derived assertion.

20.9.9.14.1. Feature Overview

Alias association keeps historical and demographic identity references navigable alongside active identifiers. The non-destructive relationship preserves how earlier information identified the person rather than rewriting that history whenever the current identity representation changes.

20.9.9.14.2. Definition / governed semantic detail

Maintain explicit associations between active identifiers and historical or demographic aliases without destructive overwriting.

20.9.9.14.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Person Identity Aliases (p. 222)	Established

20.9.9.15. Feature: Governed Identity Correction

Property	Value	Meaning
Reference Id	`FEAT-EM-04`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-04`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:120 (p. 1366)	Source standing does not approve the derived assertion.

20.9.9.15.1. Feature Overview

Governed correction applies externally authoritative identity updates and their consequences to Harmonia-managed associations. Administrative review concerns the legitimate application of the source outcome; it does not grant Harmonia an EMPI merge engine or authority to select a golden person record.

20.9.9.15.2. Definition / governed semantic detail

Apply administrative identity updates, linked-record corrections, and split/merge notifications propagated from authoritative sources.

20.9.9.15.3. Prior Candidate Derivation

v0.3 reconciliation: `BEC-44123E71` (line 2698). Working locators are not identity aliases.

20.9.9.15.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Apply Governed Person Identity Correction (p. 222)	Established
From: associated feature	Service: Person Identity Correction (p. 223)	Established

20.9.9.16. Function: Resolve Person Identifier

Property	Value	Meaning
Reference Id	`HCM-FN-resolve-person-identifier`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-RESOLVE-PERSON-IDENTIFIER`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:24 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.16.1. Definition / governed semantic detail

Validates and retrieves the authoritative demographic record bound to a specific identifier.

20.9.9.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Identifier Resolution (p. 216)	Established

20.9.9.17. Service: Person Identifier Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-person-identifier-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.SV-PERSON-IDENTIFIER-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:25 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.17.1. Definition / governed semantic detail

Exposed to Service Administration and Service Delivery to disambiguate client identities.

20.9.9.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Identifier Resolution (p. 216)	Established

20.9.9.18. Function: Correlate Person Identifiers

Property	Value	Meaning
Reference Id	`HCM-FN-correlate-person-identifiers`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-CORRELATE-PERSON-IDENTIFIERS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:27 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.18.1. Definition / governed semantic detail

Correlates local identifiers (e.g., Hospital MRN, National IHI, Pathology ID) using relationships established by authoritative information, retaining their source and namespace.

20.9.9.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Cross-Authority Identifier Correlation (p. 217)	Established

20.9.9.19. Service: Person Identifier Correlation

Property	Value	Meaning
Reference Id	`HCM-SV-person-identifier-correlation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.SV-PERSON-IDENTIFIER-CORRELATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:28 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.19.1. Definition / governed semantic detail

Exposed across the HIE to establish patient cross-reference mappings.

20.9.9.19.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Cross-Authority Identifier Correlation (p. 217)	Established

20.9.9.20. Function: Maintain Person Identity Aliases

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-person-identity-aliases`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-MAINTAIN-PERSON-IDENTITY-ALIASES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:30 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.20.1. Definition / governed semantic detail

Records and qualifies alias names, titles, and emergency aliases for a governed person identity.

20.9.9.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Identity Alias Association (p. 218)	Established

20.9.9.21. Function: Apply Governed Person Identity Correction

Property	Value	Meaning
Reference Id	`HCM-FN-apply-governed-person-identity-correction`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-APPLY-GOVERNED-PERSON-IDENTITY-CORRECTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:33 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.21.1. Definition / governed semantic detail

Processes externally authoritative corrections and merge/unlink outcomes, updates governed identity associations and propagates resulting managed-state consequences with audit history. Harmonia does not adjudicate the originating person-merge decision.

20.9.9.21.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Governed Identity Correction (p. 218)	Established
To: governed process	Process: Governed Person Identity Correction Process (p. 224)	Established

20.9.9.22. Service: Person Identity Correction

Property	Value	Meaning
Reference Id	`HCM-SV-person-identity-correction`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.SV-PERSON-IDENTITY-CORRECTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:34 (p. 1435)	Source standing does not approve the derived assertion.

20.9.9.22.1. Definition / governed semantic detail

Exposed to authorized HIM/Medical Records administrators.

20.9.9.22.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Governed Identity Correction (p. 218)	Established

20.9.9.23. Process: Governed Person Identity Correction Process

Property	Value	Meaning
Reference Id	`HCM-PR-governed-person-identity-correction-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.PR-GOVERNED-PERSON-IDENTITY-CORRECTION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:69 (p. 1415); processes:70 (p. 1415)	Source standing does not approve the derived assertion.

20.9.9.23.1. Process Overview

This Process governs the application of an externally authoritative correction rather than originating a person-merge judgement. Triage, evidence verification and approval establish that the received outcome should be applied to Harmonia's associations. The progression and change communication preserve why the managed identity account changed without making internal approval a master-person decision.

20.9.9.23.2. Definition / governed semantic detail

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Process Purpose**: Processes externally authoritative identity corrections and merge/unlink outcomes, applies their consequences to Harmonia-managed identity information and identifier associations, and communicates resulting changes with an audit history.
- **Authority boundary**: Person correction/merge decisions originate externally. Internal triage, evidence verification and approval concern applicability and governed processing of that authoritative outcome, not person matching, golden-record determination, master-person selection or originating merge adjudication.
- **State Progression Lifecycle**: Correction Requested → HIM Triage → Evidence Verified → Correction Approved → Merge/Unlink Executed → Change Broadcasted → Closed
- **Key State Dispositions**:
  - `CorrectionRequested`: Externally authoritative correction or merge/unlink outcome received for governed application.
  - `HIMTriage`: Health Information Manager reviews the received decision/outcome and its applicability.
  - `EvidenceVerified`: Authoritative source decision and supporting provenance/documentation verified.
  - `CorrectionApproved`: Governed application of the externally authoritative outcome approved; this does not originate a person-merge decision.
  - `Merge/UnlinkExecuted`: External merge/unlink consequences applied to Harmonia's managed identifier associations and audit history.
  - `ChangeBroadcasted`: Downstream clinical systems notified of merged or rectified identity.

20.9.9.23.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Apply Governed Person Identity Correction (p. 222)	Established

20.9.9.24. Feature: Identity Source & Correction Lineage

Property	Value	Meaning
Reference Id	`HCM-FT-identity-source-correction-lineage`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-STEP2-IDENTITY-SOURCE-CORRECTION-LINEAGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Person Identity Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

20.9.9.24.1. Feature Overview

Identifier correlations can change when an upstream authority corrects a relationship. Reviewers must be able to distinguish the source decision, its effective standing and the managed links affected.

20.9.9.24.2. Definition — Proposed

Retain source-attributed identity assertions and externally decided correction history.

20.9.9.24.3. Boundary

No probabilistic matching, golden-record selection or autonomous merge authority.

20.9.9.24.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Person Identity, rather than inheriting an adjacent responsibility.

20.9.9.24.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

20.9.10. BEC CT3: Healthcare Subject Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-healthcare-subject`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration	Linked responsibility references; historical records do not assert current ancestry.
Established name	Healthcare Subject	Current display rename is Proposed; meaning unchanged.
Name history	Healthcare Subject	Names do not establish another identity.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:122 (p. 1366); em:41 (p. 1436)	Source standing does not approve the derived assertion.

20.9.10.1. Capability Overview

Administer demographic and subject-of-care profile information bound to clinical records and operational activities.

20.9.10.2. Preserved Canonical semantic evidence — source context

Managing the core healthcare subject context bound to clinical records and operational activities.

20.9.10.3. Definition — Reviewed candidate

Administer demographic and subject-of-care profile information bound to clinical records and operational activities.

20.9.10.4. Boundary — retained v0.1 source explanation

Subject association does not originate clinical findings or confer authority to change the source person’s identity.

20.9.10.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Verifiable subject binding prevents otherwise valid information from being interpreted in the wrong person's care context, making it a substantive information-management responsibility.

20.9.10.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.9.10.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.9.10.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.9.10.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Subject Context Binding (p. 227)	feature FT	Canonical / Proposed
Feature: Demographic Context Governance (p. 228)	feature FT	Canonical / Proposed
Function: Establish Healthcare Subject Context (p. 229)	function	Canonical / Established
Service: Healthcare Subject Context Resolution (p. 230)	service	Canonical / Established
Function: Govern Subject Demographic Context (p. 230)	function	Canonical / Established
Service: Demographic Change Distribution (p. 231)	service	Canonical / Established

20.9.10.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.9.10.11. Feature: Subject Context Binding

Property	Value	Meaning
Reference Id	`FEAT-EM-05`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FEAT-EM-05`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Healthcare Subject Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:124 (p. 1366)	Source standing does not approve the derived assertion.

20.9.10.11.1. Feature Overview

Subject binding connects resolved identity with the records and activities concerning that person. It makes the association verifiable across encounters, orders and observations rather than assuming that proximity in a message or screen establishes the enduring subject of care.

20.9.10.11.2. Definition / governed semantic detail

Bind authenticated person identity to clinical encounters, orders, and observations as the enduring subject of care with verifiable association.

20.9.10.11.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Establish Healthcare Subject Context (p. 229)	Established
From: associated feature	Service: Healthcare Subject Context Resolution (p. 230)	Established

20.9.10.12. Feature: Demographic Context Governance

Property	Value	Meaning
Reference Id	`FEAT-EM-06`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FEAT-EM-06`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Healthcare Subject Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:125 (p. 1366)	Source standing does not approve the derived assertion.

20.9.10.12.1. Feature Overview

Demographic governance retains the provenance and temporal context of attributes used to interpret the subject of care. A changed date, status or demographic assertion can therefore be understood as sourced information rather than an unexplained overwrite of previously known context.

20.9.10.12.2. Definition / governed semantic detail

Manage essential demographic attributes (date of birth, sex, vital status) with full provenance and timestamping.

20.9.10.12.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Subject Demographic Context (p. 230)	Established
From: associated feature	Service: Demographic Change Distribution (p. 231)	Established

20.9.10.13. Function: Establish Healthcare Subject Context

Property	Value	Meaning
Reference Id	`HCM-FN-establish-healthcare-subject-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FN-ESTABLISH-HEALTHCARE-SUBJECT-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Healthcare Subject Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:45 (p. 1436)	Source standing does not approve the derived assertion.

20.9.10.13.1. Definition / governed semantic detail

Binds demographic, cultural, and communication requirements to a patient record.

20.9.10.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Subject Context Binding (p. 227)	Established

20.9.10.14. Service: Healthcare Subject Context Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-healthcare-subject-context-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.SV-HEALTHCARE-SUBJECT-CONTEXT-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Healthcare Subject Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:46 (p. 1436)	Source standing does not approve the derived assertion.

20.9.10.14.1. Definition / governed semantic detail

Provides comprehensive demographic, language, and indigenous status context to clinical applications.

20.9.10.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Subject Context Binding (p. 227)	Established

20.9.10.15. Function: Govern Subject Demographic Context

Property	Value	Meaning
Reference Id	`HCM-FN-govern-subject-demographic-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FN-GOVERN-SUBJECT-DEMOGRAPHIC-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Healthcare Subject Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:48 (p. 1436)	Source standing does not approve the derived assertion.

20.9.10.15.1. Definition / governed semantic detail

Applies validation rules to address changes, date of birth verification, and death notifications.

20.9.10.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Demographic Context Governance (p. 228)	Established

20.9.10.16. Service: Demographic Change Distribution

Property	Value	Meaning
Reference Id	`HCM-SV-demographic-change-distribution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.SV-DEMOGRAPHIC-CHANGE-DISTRIBUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Healthcare Subject Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:49 (p. 1436)	Source standing does not approve the derived assertion.

20.9.10.16.1. Definition / governed semantic detail

Emits governed subject demographic updates to subscribed clinical systems.

20.9.10.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Demographic Context Governance (p. 228)	Established

20.9.11. BEC CT3: Client Relationship Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-client-relationship`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration	Linked responsibility references; historical records do not assert current ancestry.
Established name	Client Relationship	Current display rename is Proposed; meaning unchanged.
Name history	Client Relationship	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:127 (p. 1366); em:55 (p. 1436)	Source standing does not approve the derived assertion.

20.9.11.1. Capability Overview

Administer qualified client relationships with carers, guardians, contacts and other nominated care participants.

20.9.11.2. Preserved Canonical semantic evidence — source context

Representing relationships between the client and carers, guardians, or contacts.

20.9.11.3. Definition — Reviewed candidate

Administer qualified client relationships with carers, guardians, contacts and other nominated care participants.

20.9.11.4. Boundary — retained v0.1 source explanation

Representation of a relationship does not by itself authorise every disclosure or establish a new legal mandate.

20.9.11.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Temporal relationship information materially supports appropriate care communication and context without assigning guardianship decisions to Harmonia.

20.9.11.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.9.11.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.9.11.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.9.11.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Next-of-Kin & Guardian Association (p. 233)	feature FT	Canonical / Proposed
Feature: Relationship Validity Tracking (p. 234)	feature FT	Canonical / Proposed
Function: Maintain Client Relationships (p. 235)	function	Canonical / Established
Service: Client Relationship Query (p. 235)	service	Canonical / Established
Function: Govern Client Relationship Validity (p. 236)	function	Canonical / Established
Service: Representative Authority Verification (p. 236)	service	Canonical / Established

20.9.11.10. Feature: Next-of-Kin & Guardian Association

Property	Value	Meaning
Reference Id	`FEAT-EM-07`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FEAT-EM-07`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Relationship Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:129 (p. 1366)	Source standing does not approve the derived assertion.

20.9.11.10.1. Feature Overview

This Feature records the nominated people and capacities through which a client can be contacted or represented. Association preserves who the contact or guardian is in relation to the client; it does not make every nominated person an unrestricted recipient of clinical information.

20.9.11.10.2. Definition / governed semantic detail

Associate nominated emergency contacts, legal guardians, and care agents with the client record.

20.9.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Client Relationships (p. 235)	Established
From: associated feature	Service: Client Relationship Query (p. 235)	Established

20.9.11.11. Feature: Relationship Validity Tracking

Property	Value	Meaning
Reference Id	`FEAT-EM-08`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FEAT-EM-08`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Relationship Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:130 (p. 1366)	Source standing does not approve the derived assertion.

20.9.11.11.1. Feature Overview

Relationship validity makes effective periods and authority scope part of the relationship's meaning. A contact association that was legitimate in one period or capacity need not be treated as currently valid for every purpose.

20.9.11.11.2. Definition / governed semantic detail

Maintain effective date intervals and authority scopes for nominated client relationships.

20.9.11.11.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Client Relationship Validity (p. 236)	Established
From: associated feature	Service: Representative Authority Verification (p. 236)	Established

20.9.11.12. Function: Maintain Client Relationships

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-client-relationships`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FN-MAINTAIN-CLIENT-RELATIONSHIPS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Relationship Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:59 (p. 1436)	Source standing does not approve the derived assertion.

20.9.11.12.1. Definition / governed semantic detail

Records links between patients and carers, next of kin, nominated representatives, and legal guardians.

20.9.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Next-of-Kin & Guardian Association (p. 233)	Established

20.9.11.13. Service: Client Relationship Query

Property	Value	Meaning
Reference Id	`HCM-SV-client-relationship-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.SV-CLIENT-RELATIONSHIP-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Relationship Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	em:60 (p. 1436)	Source standing does not approve the derived assertion.

20.9.11.13.1. Definition / governed semantic detail

Discloses verified support network members and contact details to clinical teams.

20.9.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Next-of-Kin & Guardian Association (p. 233)	Established

20.9.11.14. Function: Govern Client Relationship Validity

Property	Value	Meaning
Reference Id	`HCM-FN-govern-client-relationship-validity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FN-GOVERN-CLIENT-RELATIONSHIP-VALIDITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Relationship Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:62 (p. 1436)	Source standing does not approve the derived assertion.

20.9.11.14.1. Definition / governed semantic detail

Verifies power-of-attorney documents, guardianship orders, and proxy validity periods.

20.9.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Relationship Validity Tracking (p. 234)	Established

20.9.11.15. Service: Representative Authority Verification

Property	Value	Meaning
Reference Id	`HCM-SV-representative-authority-verification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.SV-REPRESENTATIVE-AUTHORITY-VERIFICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Relationship Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:63 (p. 1436)	Source standing does not approve the derived assertion.

20.9.11.15.1. Definition / governed semantic detail

Confirms legal decision-making authority for substitute consent.

20.9.11.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Relationship Validity Tracking (p. 234)	Established

20.9.12. BEC CT3: Client Privacy Guardrail Management

Property	Value	Meaning
Reference Id	`HCM-BEC-client-privacy`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration	Linked responsibility references; historical records do not assert current ancestry.
Established name	Client Privacy	Current display rename is Proposed; meaning unchanged.
Name history	Client Privacy	Names do not establish another identity.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:132 (p. 1366); em:69 (p. 1437)	Source standing does not approve the derived assertion.

20.9.12.1. Capability Overview

Manage client-associated privacy constraints, preferences, permissions, restrictions and consent directives, and evaluate their applicability to information use.

20.9.12.2. Preserved Canonical semantic evidence — source context

Enforcing client-level privacy preferences, consent directives, and confidentiality markers.

20.9.12.3. Definition — Reviewed candidate

Manage client-associated privacy constraints, preferences, permissions, restrictions and consent directives, and evaluate their applicability to information use.

20.9.12.4. Boundary — retained v0.1 source explanation

A privacy flag does not become a caller-controlled grant of authority. Consent evaluation and information access must respect established policy ownership.

20.9.12.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Consistent application of client restrictions across information pathways is a direct platform responsibility, supporting Core classification.

20.9.12.6. Boundary

Owns client-associated constraints and consent directives; general enterprise privacy policy remains outside this capability.

20.9.12.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.9.12.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.9.12.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Consent Directive Evaluation (p. 239)	feature FT	Canonical / Proposed
Feature: Confidentiality Flag Enforcement (p. 240)	feature FT	Canonical / Proposed
Function: Evaluate Client Consent (p. 240)	function	Canonical / Established
Service: Consent Decision Service (p. 241)	service	Canonical / Established
Function: Evaluate Client Confidentiality Restrictions (p. 242)	function	Canonical / Established

Element / page	Type / tier	Element / placement status
Service: Confidentiality Restriction Evaluation (p. 242)	service	Canonical / Established

20.9.12.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Digital Identity & Access Governance (p. 141)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.9.12.11. Feature: Consent Directive Evaluation

Property	Value	Meaning
Reference Id	`FEAT-EM-09`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FEAT-EM-09`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Privacy Guardrail Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:134 (p. 1366)	Source standing does not approve the derived assertion.

20.9.12.11.1. Feature Overview

Consent evaluation brings the client's applicable directives into consideration of a requested disclosure or exchange. It distinguishes the recorded directive from the particular action being evaluated, preserving the need for established policy and authority rather than treating consent data as a universal permission token.

20.9.12.11.2. Definition / governed semantic detail

Evaluate client consent directives against requested information disclosure and exchange actions.

20.9.12.11.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Evaluate Client Consent (p. 240)	Established
From: associated feature	Service: Consent Decision Service (p. 241)	Established

20.9.12.12. Feature: Confidentiality Flag Enforcement

Property	Value	Meaning
Reference Id	`FEAT-EM-10`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FEAT-EM-10`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Privacy Guardrail Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:135 (p. 1366)	Source standing does not approve the derived assertion.

20.9.12.12.1. Feature Overview

Confidentiality enforcement makes a restricted-record condition effective across discovery and presentation paths. Its purpose is consistent governed treatment of sensitive information, rather than a flag displayed in one interface while other paths disclose the same content without restriction.

20.9.12.12.2. Definition / governed semantic detail

Enforce restricted access flags (e.g., VIP, sensitive diagnosis, restricted record) across query and presentation interfaces.

20.9.12.12.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Evaluate Client Confidentiality Restrictions (p. 242)	Established
From: associated feature	Service: Confidentiality Restriction Evaluation (p. 242)	Established

20.9.12.13. Function: Evaluate Client Consent

Property	Value	Meaning
Reference Id	`HCM-FN-evaluate-client-consent`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FN-EVALUATE-CLIENT-CONSENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Privacy Guardrail Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:73 (p. 1437)	Source standing does not approve the derived assertion.

20.9.12.13.1. Definition / governed semantic detail

Evaluates patient opt-in, opt-out, and general information sharing preferences against proposed exchanges.

20.9.12.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Consent Directive Evaluation (p. 239)	Established

20.9.12.14. Service: Consent Decision Service

Property	Value	Meaning
Reference Id	`HCM-SV-consent-decision-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.SV-CONSENT-DECISION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Privacy Guardrail Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:74 (p. 1437)	Source standing does not approve the derived assertion.

20.9.12.14.1. Definition / governed semantic detail

Provides real-time consent evaluation decisions to Health Information Control.

20.9.12.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Consent Directive Evaluation (p. 239)	Established

20.9.12.15. Function: Evaluate Client Confidentiality Restrictions

Property	Value	Meaning
Reference Id	`HCM-FN-evaluate-client-confidentiality-restrictions`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FN-EVALUATE-CLIENT-CONFIDENTIALITY-RESTRICTIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Privacy Guardrail Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:76 (p. 1437)	Source standing does not approve the derived assertion.

20.9.12.15.1. Definition / governed semantic detail

Evaluates sensitive record flags (e.g., VIP masking, restricted department exclusions).

20.9.12.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Confidentiality Flag Enforcement (p. 240)	Established

20.9.12.16. Service: Confidentiality Restriction Evaluation

Property	Value	Meaning
Reference Id	`HCM-SV-confidentiality-restriction-evaluation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.SV-CONFIDENTIALITY-RESTRICTION-EVALUATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Client Administration → BEC CT3: Client Privacy Guardrail Management	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	em:77 (p. 1437)	Source standing does not approve the derived assertion.

20.9.12.16.1. Definition / governed semantic detail

Enforces record-level masking directives across the HIE.

20.9.12.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Confidentiality Flag Enforcement (p. 240)	Established

20.10. BEC CT2: Provider Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-provider-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:81 (p. 1365); enabling:137 (p. 1366); em:86 (p. 1437)	Source standing does not approve the derived assertion.

20.10.1. Capability Overview

Provider Administration maintains the practitioner, role, affiliation and engagement context needed to interpret healthcare participation. National identifiers, professional profiles, role-specific endpoints and privilege records support different questions: who the practitioner is, in what capacity they act and how they can be reached. Directory representation makes those distinctions usable without replacing the business governance that establishes professional standing.

20.10.2. Definition — Reviewed candidate

Provider Administration maintains the practitioner, role, affiliation and engagement context needed to interpret healthcare participation. National identifiers, professional profiles, role-specific endpoints and privilege records support different questions: who the practitioner is, in

what capacity they act and how they can be reached. Directory representation makes those distinctions usable without replacing the business governance that establishes professional standing.

20.10.3. Intent — retained v0.1 source explanation

Provide a trustworthy provider context for routing, participation and the application of relevant qualification and privilege constraints.

20.10.4. Boundary — retained v0.1 source explanation

Practitioner Role Resolution establishes neither Clinical Privilege nor Operational Privilege. The descriptive practitioner and role groupings do not establish reconciled CT ancestry.

20.10.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.10.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.10.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.10.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Verify Practitioner Registration (p. 254)	function	Canonical / Established
Service: Practitioner Verification Service (p. 255)	service	Canonical / Established
Function: Govern Practitioner Profile (p. 256)	function	Canonical / Established
Service: Practitioner Directory Query (p. 256)	service	Canonical / Established
Function: Maintain Practitioner Roles (p. 257)	function	Canonical / Established
Service: Practitioner Role Resolution (p. 258)	service	Canonical / Established
Function: Bind Practitioner Electronic Endpoints (p. 258)	function	Canonical / Established
Service: Practitioner Endpoint Lookup (p. 259)	service	Canonical / Established
Function: Maintain Practitioner Affiliations (p. 260)	function	Canonical / Established
Service: Practitioner Affiliation Resolution (p. 260)	service	Canonical / Established
Function: Maintain Clinical Privileges (p. 261)	function	Canonical / Established
Service: Clinical Privilege Verification (p. 262)	service	Canonical / Established
Process: Practitioner Verification Process (p. 262)	process	Canonical / Established
BEC CT3: Practitioner Profile Administration (p. 245)	capability CT3	Proposed / Proposed
BEC CT3: Practitioner Role Administration (p. 249)	capability CT3	Proposed / Proposed
BEC CT3: Vendor Liaison (p. 253)	capability CT3	Proposed / Proposed

20.10.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Workforce Management (p. 177)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.10.10. BEC CT3: Practitioner Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-practitioner`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Practitioner	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:140; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.10.10.1. Capability Overview

Administer practitioner identity, professional profile and supplied registration information with its issuing-authority attribution.

20.10.10.2. Definition — Reviewed candidate

Administer practitioner identity, professional profile and supplied registration information with its issuing-authority attribution.

20.10.10.3. Boundary

Professional registration and profile representation do not grant clinical or operational privilege.

20.10.10.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.10.10.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:140; information-responsibility.md:10

20.10.10.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.10.10.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Identifying and verifying individual healthcare professionals.

Previous overview: Practitioner identity and professional profile information let care participants be recognised across organisations. Registration and profile standing remain attributed to the issuing authority, while discovery makes that described person usable in care integration.

Previous intent: Allow a described healthcare professional to be reliably recognised and found across participating systems.

20.10.10.8. Prior Candidate Derivation

v0.3 reconciliation: `BEC-5BB55046` (line 2997). Working locators are not identity aliases.

20.10.10.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: National Practitioner Verification (p. 246)	feature FT	Canonical / Proposed
Feature: Practitioner Profile Governance (p. 247)	feature FT	Canonical / Proposed
Feature: Provider Directory Discovery & Resolution (p. 248)	feature FT	Proposed / Proposed

20.10.10.10. Feature: National Practitioner Verification

Property	Value	Meaning
Reference Id	`FEAT-EM-11`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER.FEAT-EM-11`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration → BEC CT3: Practitioner Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Provider Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:142 (p. 1366)	Source standing does not approve the derived assertion.

20.10.10.10.1. Feature Overview

National verification connects practitioner identifiers and registration information to their recognised source context. It helps distinguish an identifiable professional from an unverified directory assertion without implying that verification alone grants local clinical privileges.

20.10.10.10.2. Definition / governed semantic detail

Ingest, validate, and verify national practitioner identifiers (e.g., HPI-I) and professional registration status.

20.10.10.10.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.10.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Verify Practitioner Registration (p. 254)	Established
From: associated feature	Service: Practitioner Verification Service (p. 255)	Established

20.10.10.11. Feature: Practitioner Profile Governance

Property	Value	Meaning
Reference Id	`FEAT-EM-12`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER.FEAT-EM-12`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration → BEC CT3: Practitioner Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Provider Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:143 (p. 1366)	Source standing does not approve the derived assertion.

20.10.10.11.1. Feature Overview

Practitioner profile governance maintains the professional context used across care information and communication. Qualifications and practice status remain attributable facts, distinct from the organisation-specific role or privilege under which the practitioner may act.

20.10.10.11.2. Definition / governed semantic detail

Maintain core professional demographic details, qualifications, and active practice status.

20.10.10.11.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.10.10.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Practitioner Profile (p. 256)	Established
From: associated feature	Service: Practitioner Directory Query (p. 256)	Established

20.10.10.12. Feature: Provider Directory Discovery & Resolution

Property	Value	Meaning
Reference Id	`HCM-FT-provider-directory-discovery-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER.FEAT-STEP2-PROVIDER-DIRECTORY-DISCOVERY-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration → BEC CT3: Practitioner Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

20.10.10.12.1. Feature Overview

Routing and care participation need to find a described provider matching stated criteria, beyond merely ingesting a national identifier or updating a profile.

20.10.10.12.2. Definition — Proposed

Expose qualified practitioner discovery using identifiers, profile criteria and effective standing.

20.10.10.12.3. Boundary

Discovery supplies information; it does not determine privilege or availability.

20.10.10.12.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Practitioner, rather than inheriting an adjacent responsibility.

20.10.10.12.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

20.10.10.12.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-E9C766CB` (line 3086); `BEC-E24857D9` (line 3187). Working locators are not identity aliases.

20.10.10.12.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: associated feature	Service: Practitioner Directory Query (p. 256)	Proposed

20.10.11. BEC CT3: Practitioner Role Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-practitioner-role`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Practitioner Role	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:145; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.10.11.1. Capability Overview

Administer practitioner roles, specialties, organisational affiliations, operational capacities and recognised role endpoints.

20.10.11.2. Definition — Reviewed candidate

Administer practitioner roles, specialties, organisational affiliations, operational capacities and recognised role endpoints.

20.10.11.3. Boundary

Role, specialty, capacity and endpoint information supply context; they do not constitute a privilege determination.

20.10.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.10.11.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:145; information-responsibility.md:10

20.10.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.10.11.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Modeling the formal role, specialty, and operational capacity of a practitioner at a given organisation or location.

Previous overview: The same practitioner may act in different specialties, organisations and operational capacities. Role-specific associations and communication endpoints make that participation intelligible without converting a directory role into permission or professional privilege.

Previous intent: Make the capacity and organisational context of a practitioner interaction explicit.

20.10.11.8. Consolidated / decomposed predecessors

BEC CT3: Practitioner Role Administration (p. 249); BEC CT3: Provider Relationship (p. 925)

20.10.11.9. Prior Candidate Derivation

v0.3 reconciliation: `BEC-2DEC073A` (line 3099). Working locators are not identity aliases.

20.10.11.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Role & Specialty Modeling (p. 251)	feature FT	Canonical / Proposed
Feature: Electronic Endpoint Binding (p. 252)	feature FT	Canonical / Proposed
Feature: Practitioner-Organisation Affiliation (p. 252)	feature FT	Canonical / Proposed

20.10.11.11. Feature: Role & Specialty Modeling

Property	Value	Meaning
Reference Id	`FEAT-EM-13`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE.FEAT-EM-13`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration → BEC CT3: Practitioner Role Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Provider Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:147 (p. 1366)	Source standing does not approve the derived assertion.

20.10.11.11.1. Feature Overview

Role and specialty modelling describes the capacity in which a practitioner participates at an organisation or location. This keeps the person separate from each acting role and its service context; role resolution itself establishes neither Clinical Privilege nor Operational Privilege.

20.10.11.11.2. Definition / governed semantic detail

Define and associate clinical specialties, practice roles, and service capacities for a practitioner.

20.10.11.11.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.10.11.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Practitioner Roles (p. 257)	Established
From: associated feature	Service: Practitioner Role Resolution (p. 258)	Established

20.10.11.12. Feature: Electronic Endpoint Binding

Property	Value	Meaning
Reference Id	`FEAT-EM-14`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE.FEAT-EM-14`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration → BEC CT3: Practitioner Role Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Provider Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:148 (p. 1366)	Source standing does not approve the derived assertion.

20.10.11.12.1. Feature Overview

Endpoint binding makes a verified communication destination specific to the practitioner's acting role. It supports routing to the intended professional capacity rather than assuming that every address attached to the person is interchangeable across organisations and services.

20.10.11.12.2. Definition / governed semantic detail

Associate validated communication endpoints (secure messaging addresses, direct routing queues) with specific practitioner roles.

20.10.11.12.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.10.11.12.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Bind Practitioner Electronic Endpoints (p. 258)	Established
From: associated feature	Service: Practitioner Endpoint Lookup (p. 259)	Established

20.10.11.13. Feature: Practitioner-Organisation Affiliation

Property	Value	Meaning
Reference Id	`FEAT-EM-15`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE.FEAT-EM-15`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration → BEC CT3: Practitioner Role Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-RELATIONSHIP.FEAT-EM-15`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Provider Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:152 (p. 1366)	Source standing does not approve the derived assertion.

20.10.11.13.1. Feature Overview

Affiliation records keep practitioner participation in an organisation temporally intelligible. They support interpretation of an actor's organisational context while preserving that affiliation, employment, qualification and permission to undertake a particular activity answer different questions.

20.10.11.13.2. Definition / governed semantic detail

Maintain temporal affiliations between practitioners and healthcare organizations.

20.10.11.13.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.10.11.13.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Practitioner Affiliations (p. 260)	Established
From: associated feature	Service: Practitioner Affiliation Resolution (p. 260)	Established

20.10.12. BEC CT3: Vendor Liaison

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-vendor-liaison`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-VENDOR-LIAISON`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

20.10.12.1. Capability Overview

Maintain the information and liaison arrangements needed to interact with vendors supporting healthcare provision. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

20.10.12.2. Definition — Reviewed candidate

Maintain the information and liaison arrangements needed to interact with vendors supporting healthcare provision.

20.10.12.3. Intent

Maintain the information and liaison arrangements needed to interact with vendors supporting healthcare provision.

20.10.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

20.10.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.10.13. Function: Verify Practitioner Registration

Property	Value	Meaning
Reference Id	`HCM-FN-verify-practitioner-registration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-VERIFY-PRACTITIONER-REGISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:92 (p. 1437)	Source standing does not approve the derived assertion.

20.10.13.1. Definition / governed semantic detail

Validates national registration, specialty endorsements, and disciplinary sanctions against regulatory registries (e.g., AHPRA).

20.10.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: National Practitioner Verification (p. 246)	Established
To: governed process	Process: Practitioner Verification Process (p. 262)	Established

20.10.14. Service: Practitioner Verification Service

Property	Value	Meaning
Reference Id	`HCM-SV-practitioner-verification-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-VERIFICATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:93 (p. 1437)	Source standing does not approve the derived assertion.

20.10.14.1. Definition / governed semantic detail

Discloses verified registration and status to credentialing and clinical systems.

20.10.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: National Practitioner Verification (p. 246)	Established

20.10.15. Function: Govern Practitioner Profile

Property	Value	Meaning
Reference Id	`HCM-FN-govern-practitioner-profile`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-GOVERN-PRACTITIONER-PROFILE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:96 (p. 1437)	Source standing does not approve the derived assertion.

20.10.15.1. Definition / governed semantic detail

Maintains official names, professional identifiers (e.g., HPI-I, Prescriber Number), and clinical qualifications.

20.10.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Practitioner Profile Governance (p. 247)	Established

20.10.16. Service: Practitioner Directory Query

Property	Value	Meaning
Reference Id	`HCM-SV-practitioner-directory-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-DIRECTORY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:97 (p. 1437)	Source standing does not approve the derived assertion.

20.10.16.1. Definition / governed semantic detail

Provides provider directory lookups across the healthcare network.

20.10.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Practitioner Profile Governance (p. 247)	Established
To: associated feature	Feature: Provider Directory Discovery & Resolution (p. 248)	Proposed

20.10.17. Function: Maintain Practitioner Roles

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-practitioner-roles`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-MAINTAIN-PRACTITIONER-ROLES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:99 (p. 1437)	Source standing does not approve the derived assertion.

20.10.17.1. Definition / governed semantic detail

Assigns and maintains clinical roles (e.g., Specialist Physician, Registrar, General Practitioner) fulfilled by a practitioner.

20.10.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Role & Specialty Modeling (p. 251)	Established

20.10.18. Service: Practitioner Role Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-practitioner-role-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-ROLE-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:100 (p. 1437)	Source standing does not approve the derived assertion.

20.10.18.1. Definition / governed semantic detail

Resolves the specific clinical capacities of a practitioner.

20.10.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Role & Specialty Modeling (p. 251)	Established

20.10.19. Function: Bind Practitioner Electronic Endpoints

Property	Value	Meaning
Reference Id	`HCM-FN-bind-practitioner-electronic-endpoints`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-BIND-PRACTITIONER-ELECTRONIC-ENDPOINTS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:102 (p. 1437)	Source standing does not approve the derived assertion.

20.10.19.1. Definition / governed semantic detail

Associates secure messaging addresses, direct communication endpoints, and notification preferences to practitioner roles.

20.10.19.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Electronic Endpoint Binding (p. 252)	Established

20.10.20. Service: Practitioner Endpoint Lookup

Property	Value	Meaning
Reference Id	`HCM-SV-practitioner-endpoint-lookup`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-ENDPOINT-LOOKUP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:103 (p. 1437)	Source standing does not approve the derived assertion.

20.10.20.1. Definition / governed semantic detail

Provides secure message routing destinations for clinical communication.

20.10.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Electronic Endpoint Binding (p. 252)	Established

20.10.21. Function: Maintain Practitioner Affiliations

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-practitioner-affiliations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-MAINTAIN-PRACTITIONER-AFFILIATIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:105 (p. 1437)	Source standing does not approve the derived assertion.

20.10.21.1. Definition / governed semantic detail

Tracks employment, admitting rights, and consulting appointments at specific healthcare facilities.

20.10.21.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Practitioner-Organisation Affiliation (p. 252)	Established

20.10.22. Service: Practitioner Affiliation Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-practitioner-affiliation-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-AFFILIATION-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	em:106 (p. 1437)	Source standing does not approve the derived assertion.

20.10.22.1. Definition / governed semantic detail

Discloses organizational ties and practice locations.

20.10.22.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Practitioner-Organisation Affiliation (p. 252)	Established

20.10.23. Function: Maintain Clinical Privileges

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-clinical-privileges`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-MAINTAIN-CLINICAL-PRIVILEGES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:108 (p. 1437)	Source standing does not approve the derived assertion.

20.10.23.1. Definition / governed semantic detail

Records approved clinical scopes of practice, procedural authorizations, and prescribing privileges.

20.10.23.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinical Privileging Record (p. 625)	Established

20.10.24. Service: Clinical Privilege Verification

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-privilege-verification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-CLINICAL-PRIVILEGE-VERIFICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:109 (p. 1437)	Source standing does not approve the derived assertion.

20.10.24.1. Definition / governed semantic detail

Verifies procedural and prescribing authority for clinical ordering systems.

20.10.24.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Clinical Privileging Record (p. 625)	Established

20.10.25. Process: Practitioner Verification Process

Property	Value	Meaning
Reference Id	`HCM-PR-practitioner-verification-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.PR-PRACTITIONER-VERIFICATION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:85 (p. 1416); processes:86 (p. 1416)	Source standing does not approve the derived assertion.

20.10.25.1. Process Overview

Verification progresses from checking the recognised professional source to the locally applicable endorsement and continuing verification status. The distinction matters because a nationally identifiable practitioner is not automatically privileged for every service activity. The illustrated progression retains the health service's endorsement responsibility rather than letting directory ingestion grant clinical authority.

20.10.25.2. Definition / governed semantic detail

- **Owning Capability**: `ProviderAdministration`
- **Process Purpose**: Governs the lifecycle of validating, certifying, and periodically re-verifying a healthcare practitioner's professional credentials, registration status, and clinical scope of practice.
- **State Progression Lifecycle**: Verification Initiated → Regulatory Registry Queried → Credentials Validated → Privileges Endorsed → Active Verified → Expired / Suspended
- **Key State Dispositions**:
  - `VerificationInitiated`: New practitioner registration or periodic re-credentialing triggered.
  - `RegulatoryRegistryQueried`: Real-time query executed against national registry (e.g., AHPRA).
  - `CredentialsValidated`: Primary qualifications, specialist endorsements, and sanctions checked.
  - `PrivilegesEndorsed`: Health service medical administration endorses admitting/procedural scope.
  - `ActiveVerified`: Practitioner verified for clinical ordering and electronic communication.
  - `Expired/Suspended`: Verification lapses or regulatory sanction triggers privilege suspension.

20.10.25.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Verify Practitioner Registration (p. 254)	Established

20.11. BEC CT2: Organisation Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-organisation-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:86 (p. 1365); enabling:158 (p. 1367); em:118 (p. 1438)	Source standing does not approve the derived assertion.

20.11.1. Capability Overview

Organisation Administration maintains the identity, profiles, structural relationships and contact context of participating healthcare organisations. It enables a legal or operational entity to be distinguished from a department, network relationship or communication channel. This supports consistent interpretation of providers and destinations across information flows rather than allowing each interface to create an incompatible organisational account.

20.11.2. Definition — Reviewed candidate

Organisation Administration maintains the identity, profiles, structural relationships and contact context of participating healthcare organisations. It enables a legal or operational entity to be distinguished from a department, network relationship or communication channel. This supports consistent interpretation of providers and destinations across information flows rather than allowing each interface to create an incompatible organisational account.

20.11.3. Boundary — retained v0.1 source explanation

Representing organisational structure does not acquire corporate authority or ownership of every service offered by the organisation.

20.11.4. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.11.5. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Verify Healthcare Organisation (p. 270)	function	Canonical / Established
Service: Organisation Verification Service (p. 271)	service	Canonical / Established
Function: Govern Organisation Profile (p. 272)	function	Canonical / Established

Element / page	Type / tier	Element / placement status
Service: Organisation Directory Query (p. 272)	service	Canonical / Established
Function: Maintain Organisational Structure (p. 273)	function	Canonical / Established
Service: Organisational Hierarchy Resolution (p. 274)	service	Canonical / Established
Function: Maintain Organisation Contacts (p. 274)	function	Canonical / Established
Service: Organisation Contact Lookup (p. 275)	service	Canonical / Established
BEC CT3: Healthcare Organisation Profile Administration (p. 265)	capability CT3	Proposed / Proposed

20.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.11.9. BEC CT3: Healthcare Organisation Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-healthcare-organisation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Healthcare Organisation	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:161; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.11.9.1. Capability Overview

Administer identifying, descriptive, structural, location, contact, registered/legal and recognised endpoint information needed to represent and interact with a healthcare organisation.

20.11.9.2. Definition — Reviewed candidate

Administer identifying, descriptive, structural, location, contact, registered/legal and recognised endpoint information needed to represent and interact with a healthcare organisation.

20.11.9.3. Boundary

Maintains verified legal and operational identity information without corporate management authority.

20.11.9.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.11.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:161; information-responsibility.md:10

20.11.9.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.11.9.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Authoritative identification and profiling of legal and operational healthcare entities.

Previous overview: An identifiable legal or operational healthcare organisation needs verified identifiers, a profile and published standing. This description anchors service and affiliation relationships without managing the organisation itself.

Previous intent: Give service and affiliation references a verified organisational identity anchor.

20.11.9.8. Consolidated / decomposed predecessors

BEC CT3: Healthcare Organisation Profile Administration (p. 265); BEC CT3: Organisational Structure (p. 927); BEC CT3: Organisation Contact (p. 928)

20.11.9.9. Prior Candidate Derivation

v0.3 reconciliation: `BEC-ACE86B9B` (line 3348). Working locators are not identity aliases.

20.11.9.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: National Organisation Verification (p. 267)	feature FT	Canonical / Proposed
Feature: Organisation Profile Management (p. 268)	feature FT	Canonical / Proposed
Feature: Hierarchical Structure Navigation (p. 268)	feature FT	Canonical / Proposed
Feature: Directory Contact Management (p. 269)	feature FT	Canonical / Proposed

20.11.9.11. Feature: National Organisation Verification

Property	Value	Meaning
Reference Id	`FEAT-EM-17`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-17`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration → BEC CT3: Healthcare Organisation Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Organisation Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:163 (p. 1367)	Source standing does not approve the derived assertion.

20.11.9.11.1. Feature Overview

National organisation verification grounds an organisation identifier and entity classification in their recognised context. This supports reliable interpretation of the healthcare participant without assuming that its identity establishes every service, location or authority associated with it.

20.11.9.11.2. Definition / governed semantic detail

Validate national organization identifiers (e.g., HPI-O) and legal entity classifications.

20.11.9.11.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.11.9.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Verify Healthcare Organisation (p. 270)	Established
From: associated feature	Service: Organisation Verification Service (p. 271)	Established

20.11.9.12. Feature: Organisation Profile Management

Property	Value	Meaning
Reference Id	`FEAT-EM-18`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-18`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration → BEC CT3: Healthcare Organisation Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Organisation Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:164 (p. 1367)	Source standing does not approve the derived assertion.

20.11.9.12.1. Feature Overview

Organisation profiles provide a governed account of the participant's status and published service descriptions. They support consistent directory use across systems while leaving the organisation's real-world governance and the definition of individual offerings distinguishable.

20.11.9.12.2. Definition / governed semantic detail

Govern directory profiles, operational status, and published service descriptions.

20.11.9.12.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.11.9.12.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Organisation Profile (p. 272)	Established
From: associated feature	Service: Organisation Directory Query (p. 272)	Established

20.11.9.13. Feature: Hierarchical Structure Navigation

Property	Value	Meaning
Reference Id	`FEAT-EM-19`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-19`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration → BEC CT3: Healthcare Organisation Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATIONAL-STRUCTURE.FEAT-EM-19`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Organisation Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:168 (p. 1367)	Source standing does not approve the derived assertion.

20.11.9.13.1. Feature Overview

Hierarchy navigation makes departmental, divisional and network relationships usable without flattening them into unrelated organisations. Traversal reflects established structure; it does not create corporate authority or infer responsibility from the way a directory happens to display a tree.

20.11.9.13.2. Definition / governed semantic detail

Model, traverse, and resolve nested departmental, divisional, and regional healthcare networks.

20.11.9.13.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.11.9.13.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Organisational Structure (p. 273)	Established
From: associated feature	Service: Organisational Hierarchy Resolution (p. 274)	Established

20.11.9.14. Feature: Directory Contact Management

Property	Value	Meaning
Reference Id	`FEAT-EM-20`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-20`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration → BEC CT3: Healthcare Organisation Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATION-CONTACT.FEAT-EM-20`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Organisation Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:172 (p. 1367)	Source standing does not approve the derived assertion.

20.11.9.14.1. Feature Overview

Contact management provides verified administrative and clinical communication channels for organisations. It supports reaching the appropriate participant while distinguishing a directory contact from the role-specific destination and authority relevant to a particular clinical exchange.

20.11.9.14.2. Definition / governed semantic detail

Manage verified phone, email, and administrative contact channels for healthcare facilities.

20.11.9.14.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.11.9.14.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Organisation Contacts (p. 274)	Established
From: associated feature	Service: Organisation Contact Lookup (p. 275)	Established

20.11.10. Function: Verify Healthcare Organisation

Property	Value	Meaning
Reference Id	`HCM-FN-verify-healthcare-organisation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-VERIFY-HEALTHCARE-ORGANISATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:121 (p. 1438)	Source standing does not approve the derived assertion.

20.11.10.1. Definition / governed semantic detail

Validates enterprise legal identity, national facility identifiers (e.g., HPI-O), and accreditation status.

20.11.10.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: National Organisation Verification (p. 267)	Established

20.11.11. Service: Organisation Verification Service

Property	Value	Meaning
Reference Id	`HCM-SV-organisation-verification-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATION-VERIFICATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:122 (p. 1438)	Source standing does not approve the derived assertion.

20.11.11.1. Definition / governed semantic detail

Provides verified organisation credentials to external and internal partners.

20.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: National Organisation Verification (p. 267)	Established

20.11.12. Function: Govern Organisation Profile

Property	Value	Meaning
Reference Id	`HCM-FN-govern-organisation-profile`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-GOVERN-ORGANISATION-PROFILE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:124 (p. 1438)	Source standing does not approve the derived assertion.

20.11.12.1. Definition / governed semantic detail

Maintains official trading names, enterprise registration numbers, and governing bodies.

20.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Organisation Profile Management (p. 268)	Established

20.11.13. Service: Organisation Directory Query

Property	Value	Meaning
Reference Id	`HCM-SV-organisation-directory-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATION-DIRECTORY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:125 (p. 1438)	Source standing does not approve the derived assertion.

20.11.13.1. Definition / governed semantic detail

Discloses healthcare organisation profiles across the region.

20.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Organisation Profile Management (p. 268)	Established

20.11.14. Function: Maintain Organisational Structure

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-organisational-structure`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-MAINTAIN-ORGANISATIONAL-STRUCTURE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:127 (p. 1438)	Source standing does not approve the derived assertion.

20.11.14.1. Definition / governed semantic detail

Models parent-subsidiary relationships, clinical directorates, departments, and operational divisions.

20.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Hierarchical Structure Navigation (p. 268)	Established

20.11.15. Service: Organisational Hierarchy Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-organisational-hierarchy-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATIONAL-HIERARCHY-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:128 (p. 1438)	Source standing does not approve the derived assertion.

20.11.15.1. Definition / governed semantic detail

Resolves department nesting and reporting hierarchies.

20.11.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Hierarchical Structure Navigation (p. 268)	Established

20.11.16. Function: Maintain Organisation Contacts

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-organisation-contacts`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-MAINTAIN-ORGANISATION-CONTACTS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:130 (p. 1438)	Source standing does not approve the derived assertion.

20.11.16.1. Definition / governed semantic detail

Manages administrative, clinical, and emergency contact details for facilities.

20.11.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Directory Contact Management (p. 269)	Established

20.11.17. Service: Organisation Contact Lookup

Property	Value	Meaning
Reference Id	`HCM-SV-organisation-contact-lookup`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATION-CONTACT-LOOKUP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:131 (p. 1438)	Source standing does not approve the derived assertion.

20.11.17.1. Definition / governed semantic detail

Provides official operational contact details.

20.11.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Directory Contact Management (p. 269)	Established

20.12. BEC CT2: Location Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-location-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:90 (p. 1365); enabling:174 (p. 1367); em:140 (p. 1439)	Source standing does not approve the derived assertion.

20.12.1. Capability Overview

Location Administration represents the places and physical relationships relevant to healthcare activity, from facilities to particular care places. Address and geospatial context answer where a place is; hierarchy explains its surroundings; care-place definition records the local characteristics relevant to care. These are stable descriptive responsibilities, distinct from the changing operational state of a bed or ward.

20.12.2. Definition — Reviewed candidate

Location Administration represents the places and physical relationships relevant to healthcare activity, from facilities to particular care places. Address and geospatial context answer where a place is; hierarchy explains its surroundings; care-place definition records the local characteristics relevant to care. These are stable descriptive responsibilities, distinct from the changing operational state of a bed or ward.

20.12.3. Boundary — retained v0.1 source explanation

Defining a place does not establish occupancy, readiness, facilities maintenance or service capacity.

20.12.4. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.12.5. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.12.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Maintain Healthcare Location (p. 283)	function	Canonical / Established
Service: Location Directory Query (p. 284)	service	Canonical / Established
Function: Maintain Physical Location Hierarchy (p. 284)	function	Canonical / Established
Service: Location Hierarchy Resolution (p. 285)	service	Canonical / Established
Function: Maintain Care-Place Definition (p. 286)	function	Canonical / Established
Service: Care-Place Specification Lookup (p. 286)	service	Canonical / Established
BEC CT3: Healthcare Location Profile Administration (p. 277)	capability CT3	Proposed / Proposed
BEC CT3: Care-Place Definition Administration (p. 281)	capability CT3	Proposed / Proposed

20.12.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.12.9. BEC CT3: Healthcare Location Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-healthcare-location`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Healthcare Location	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:177; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.12.9.1. Capability Overview

Administer the identity, address, geospatial description and physical containment relationships of healthcare locations.

20.12.9.2. Definition — Reviewed candidate

Administer the identity, address, geospatial description and physical containment relationships of healthcare locations.

20.12.9.3. Boundary

Location description does not establish occupancy, cleanliness, service capacity or service readiness.

20.12.9.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.12.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:177; information-responsibility.md:10

20.12.9.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.12.9.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Identifying and profiling geographic locations, physical campuses, and health facilities.

Previous overview: Addresses, geospatial context and facility identity establish which physical place an information or activity concern refers to. Describing a location remains separate from deciding what services can operate there now.

Previous intent: Identify the physical place relevant to care information and operational activity.

20.12.9.8. Consolidated / decomposed predecessors

BEC CT3: Healthcare Location Profile Administration (p. 277); BEC CT3: Location Structure (p. 930)

20.12.9.9. Prior Candidate Derivation

v0.3 reconciliation: `BEC-7E83F611` (line 3639). Working locators are not identity aliases.

20.12.9.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Location Geospatial & Address Resolution (p. 279)	feature FT	Canonical / Proposed
Feature: Physical Hierarchy Modeling (p. 280)	feature FT	Canonical / Proposed

20.12.9.11. Feature: Location Geospatial & Address Resolution

Property	Value	Meaning
Reference Id	`FEAT-EM-21`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION.FEAT-EM-21`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration → BEC CT3: Healthcare Location Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Location Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:179 (p. 1367)	Source standing does not approve the derived assertion.

20.12.9.11.1. Feature Overview

Address and geospatial resolution gives a recognised healthcare location an intelligible physical reference. It supports place interpretation without treating a valid address or recorded operating status as proof that a particular service or care place is currently ready.

20.12.9.11.2. Definition / governed semantic detail

Validate physical addresses, geospatial coordinates, and location operational status.

20.12.9.11.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.12.9.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Healthcare Location (p. 283)	Established
From: associated feature	Service: Location Directory Query (p. 284)	Established

20.12.9.12. Feature: Physical Hierarchy Modeling

Property	Value	Meaning
Reference Id	`FEAT-EM-22`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION.FEAT-EM-22`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration → BEC CT3: Healthcare Location Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-LOCATION-STRUCTURE.FEAT-EM-22`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Location Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:183 (p. 1367)	Source standing does not approve the derived assertion.

20.12.9.12.1. Feature Overview

Physical hierarchy modelling relates a care environment to its campus, building, floor or ward context. The relationships describe where places sit physically; they do not automatically reproduce organisational hierarchy, service responsibility or operational capacity.

20.12.9.12.2. Definition / governed semantic detail

Represent campus, building, wing, floor, and ward architectural relationships.

20.12.9.12.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.12.9.12.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Physical Location Hierarchy (p. 284)	Established
From: associated feature	Service: Location Hierarchy Resolution (p. 285)	Established

20.12.10. BEC CT3: Care-Place Definition Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-care-place-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-CARE-PLACE-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Care-Place Definition	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:185; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.12.10.1. Capability Overview

Administer the stable description, characteristics and physical location of a bed, bay, chair or other care-place.

20.12.10.2. Definition — Reviewed candidate

Administer the stable description, characteristics and physical location of a bed, bay, chair or other care-place.

20.12.10.3. Boundary

Describes a place and its capabilities; operational bed state and readiness remain separately managed.

20.12.10.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.12.10.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:185; information-responsibility.md:10

20.12.10.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.12.10.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Modeling the atomic care delivery places (beds, bays, treatment chairs, surgical tables).

Previous overview: A particular bed, bay, treatment chair or procedural place has stable physical characteristics relevant to care. Those descriptive capabilities remain separate from occupancy, cleaning state and suitability for a current patient.

Previous intent: Describe the stable characteristics that distinguish one care place from another.

20.12.10.8. Prior Candidate Derivation

v0.3 reconciliation: `BEC-0C8D2717` (line 3791). Working locators are not identity aliases.

20.12.10.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Atomic Care-Place Modeling (p. 282)	feature FT	Canonical / Proposed

20.12.10.10. Feature: Atomic Care-Place Modeling

Property	Value	Meaning
Reference Id	`FEAT-EM-23`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-CARE-PLACE-DEFINITION.FEAT-EM-23`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration → BEC CT3: Care-Place Definition Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Established broader owner	BEC CT2: Location Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:187 (p. 1367)	Source standing does not approve the derived assertion.

20.12.10.10.1. Feature Overview

Care-place modelling identifies the particular physical place and characteristics relevant to care, such as isolation or telemetry capability. It supplies the descriptive frame against which changing occupancy and readiness can be considered without making those states part of the place's identity.

20.12.10.10.2. Definition / governed semantic detail

Define specific beds, care-bays, and examination rooms with their physical capabilities (e.g., telemetry-capable, negative-pressure).

20.12.10.10.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.12.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Care-Place Definition (p. 286)	Established
From: associated feature	Service: Care-Place Specification Lookup (p. 286)	Established

20.12.11. Function: Maintain Healthcare Location

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-healthcare-location`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.FN-MAINTAIN-HEALTHCARE-LOCATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:145 (p. 1439)	Source standing does not approve the derived assertion.

20.12.11.1. Definition / governed semantic detail

Records physical sites, campuses, outpatient clinics, and community health centres.

20.12.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Location Geospatial & Address Resolution (p. 279)	Established

20.12.12. Service: Location Directory Query

Property	Value	Meaning
Reference Id	`HCM-SV-location-directory-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.SV-LOCATION-DIRECTORY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:146 (p. 1439)	Source standing does not approve the derived assertion.

20.12.12.1. Definition / governed semantic detail

Exposes location profiles and geospatial addresses.

20.12.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Location Geospatial & Address Resolution (p. 279)	Established

20.12.13. Function: Maintain Physical Location Hierarchy

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-physical-location-hierarchy`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.FN-MAINTAIN-PHYSICAL-LOCATION-HIERARCHY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:148 (p. 1439)	Source standing does not approve the derived assertion.

20.12.13.1. Definition / governed semantic detail

Models the nested structural containment tree: `Campus` → `Building` → `Floor` → `Ward/Suite` → `Room/Bay` → `BedCare-Place`.

20.12.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Physical Hierarchy Modeling (p. 280)	Established

20.12.14. Service: Location Hierarchy Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-location-hierarchy-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.SV-LOCATION-HIERARCHY-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:149 (p. 1439)	Source standing does not approve the derived assertion.

20.12.14.1. Definition / governed semantic detail

Discloses physical containment and navigation paths for clinical facilities.

20.12.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Physical Hierarchy Modeling (p. 280)	Established

20.12.15. Function: Maintain Care-Place Definition

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-care-place-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.FN-MAINTAIN-CARE-PLACE-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:151 (p. 1439)	Source standing does not approve the derived assertion.

20.12.15.1. Definition / governed semantic detail

Defines static bed bays, treatment chairs, surgical tables, and emergency resuscitation cubicles, including equipment capabilities and oxygen/telemetry infrastructure.

20.12.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Atomic Care-Place Modeling (p. 282)	Established

20.12.16. Service: Care-Place Specification Lookup

Property	Value	Meaning
Reference Id	`HCM-SV-care-place-specification-lookup`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.SV-CARE-PLACE-SPECIFICATION-LOOKUP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:152 (p. 1439)	Source standing does not approve the derived assertion.

20.12.16.1. Definition / governed semantic detail

Discloses care-place physical capabilities to Bed & Care-Place Management.

20.12.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Atomic Care-Place Modeling (p. 282)	Established

20.13. BEC CT2: Health Service Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-health-service-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:94 (p. 1365); enabling:189 (p. 1368); em:161 (p. 1439)	Source standing does not approve the derived assertion.

20.13.1. Capability Overview

Health Service Administration represents identifiable services and the conditions under which organisations, locations and practitioner roles provide them. Catalogue publishing, service-location-provider association, operating schedules and eligibility definitions allow a service to be understood as more than a location or specialty label. The service context remains material wherever activity meaning or accountability depends on it.

20.13.2. Definition — Reviewed candidate

Health Service Administration represents identifiable services and the conditions under which organisations, locations and practitioner roles provide them. Catalogue publishing, service-location-provider association, operating schedules and eligibility definitions allow a service to be understood as more than a location or specialty label. The service context remains material wherever activity meaning or accountability depends on it.

20.13.3. Boundary — retained v0.1 source explanation

This capability owns the service mapping associations, not the mapped organisations, locations or practitioners. The supplied Domain03-to-Domain04 trace preserves that distinction; it does not transfer entity ownership.

20.13.4. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.13.5. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.13.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.13.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Maintain Healthcare Service Definition (p. 294)	function	Canonical / Established
Service: Healthcare Service Directory Query (p. 295)	service	Canonical / Established
Function: Maintain Service / Location / Provider Map (p. 296)	function	Canonical / Established
Service: Service Provision Resolution (p. 296)	service	Canonical / Established
Function: Maintain Service Availability (p. 297)	function	Canonical / Established
Service: Service Availability Query (p. 298)	service	Canonical / Established
Function: Maintain Service Eligibility Rules (p. 298)	function	Canonical / Established
Service: Service Eligibility Evaluation (p. 299)	service	Canonical / Established
BEC CT3: Healthcare Service Profile Administration (p. 289)	capability CT3	Proposed / Proposed

20.13.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.13.9. BEC CT3: Healthcare Service Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-healthcare-service-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Healthcare Service Definition	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:192; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.13.9.1. Capability Overview

Administer service definitions, provision relationships, defined availability and eligibility information describing offered healthcare services.

20.13.9.2. Definition — Reviewed candidate

Administer service definitions, provision relationships, defined availability and eligibility information describing offered healthcare services.

20.13.9.3. Boundary

Defined availability is not dynamic operational capacity, collective readiness or authority to accept clinical care.

20.13.9.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.13.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:192; information-responsibility.md:10

20.13.9.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.13.9.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Specifying clinical and administrative health services offered to the public or clinical referrers.

Previous overview: An offered service has an identifiable clinical or administrative purpose, specialty and published intake description. That definition is distinct from the provider/location binding through which the service can be delivered.

Previous intent: Make the purpose and published offering of a healthcare service recognisable independently of its current delivery arrangement.

Previous boundary: Defines what is offered; offering existence does not establish current availability or collective readiness.

20.13.9.8. Consolidated / decomposed predecessors

BEC CT3: Healthcare Service Profile Administration (p. 289); BEC CT3: Service Provision Relationship (p. 931); BEC CT3: Service Availability Definition (p. 932); BEC CT3: Service Eligibility Definition (p. 933)

20.13.9.9. Prior Candidate Derivation

v0.3 reconciliation: `BEC-1864D570` (line 3872). Working locators are not identity aliases.

20.13.9.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Service Catalogue Publishing (p. 291)	feature FT	Canonical / Proposed
Feature: Service-Location-Provider Binding (p. 291)	feature FT	Canonical / Proposed
Feature: Operating Schedule Specification (p. 292)	feature FT	Canonical / Proposed
Feature: Eligibility & Catchment Specification (p. 293)	feature FT	Canonical / Proposed

20.13.9.11. Feature: Service Catalogue Publishing

Property	Value	Meaning
Reference Id	`FEAT-EM-24`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-24`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration → BEC CT3: Healthcare Service Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Health Service Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:194 (p. 1368)	Source standing does not approve the derived assertion.

20.13.9.11.1. Feature Overview

Service catalogue publishing makes identifiable service definitions and intake context available to regional participants. A published service can be discovered as an offering rather than inferred from an organisation's name; publication alone does not establish present availability or collective readiness.

20.13.9.11.2. Definition / governed semantic detail

Publish structured healthcare service definitions, clinical specialties, and intake criteria to regional directories.

20.13.9.11.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.13.9.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Healthcare Service Definition (p. 294)	Established
From: associated feature	Service: Healthcare Service Directory Query (p. 295)	Established

20.13.9.12. Feature: Service-Location-Provider Binding

Property	Value	Meaning
Reference Id	`FEAT-EM-25`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-25`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration → BEC CT3: Healthcare Service Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-PROVISION-RELATIONSHIP.FEAT-EM-25`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Service Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:198 (p. 1368)	Source standing does not approve the derived assertion.

20.13.9.12.1. Feature Overview

Service-location-provider binding explains which participant provides an identifiable service in which place and acting capacities. The mapping associations belong to Health Service Administration, while the organisations, locations and practitioner entities retain their own information responsibility.

20.13.9.12.2. Definition / governed semantic detail

Correlate which organization provides a named health service at which physical location with which practitioner roles.

20.13.9.12.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.13.9.12.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Service / Location / Provider Map (p. 296)	Established
From: associated feature	Service: Service Provision Resolution (p. 296)	Established

20.13.9.13. Feature: Operating Schedule Specification

Property	Value	Meaning
Reference Id	`FEAT-EM-26`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-26`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration → BEC CT3: Healthcare Service Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-AVAILABILITY-DEFINITION.FEAT-EM-26`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Service Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:202 (p. 1368)	Source standing does not approve the derived assertion.

20.13.9.13.1. Feature Overview

Operating schedules make temporal availability and exceptions explicit for a service. They distinguish an existing offering from one available at the relevant time, while leaving actual resource readiness and operational capacity as separate concerns.

20.13.9.13.2. Definition / governed semantic detail

Define temporal availability, clinic operating hours, and on-call service windows.

20.13.9.13.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.13.9.13.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Service Availability (p. 297)	Established
From: associated feature	Service: Service Availability Query (p. 298)	Established

20.13.9.14. Feature: Eligibility & Catchment Specification

Property	Value	Meaning
Reference Id	`FEAT-EM-27`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-27`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration → BEC CT3: Healthcare Service Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-ELIGIBILITY-DEFINITION.FEAT-EM-27`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Service Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:206 (p. 1368)	Source standing does not approve the derived assertion.

20.13.9.14.1. Feature Overview

Eligibility specification captures the clinical thresholds and geographical bounds applicable to service access. It provides the criteria context needed to consider a request without making publication of those criteria an individual eligibility decision or a completed allocation.

20.13.9.14.2. Definition / governed semantic detail

Define clinical referral thresholds and geographic catchment boundaries.

20.13.9.14.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.13.9.14.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Service Eligibility Rules (p. 298)	Established
From: associated feature	Service: Service Eligibility Evaluation (p. 299)	Established

20.13.10. Function: Maintain Healthcare Service Definition

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-healthcare-service-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-HEALTHCARE-SERVICE-DEFINITION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:174 (p. 1439)	Source standing does not approve the derived assertion.

20.13.10.1. Definition / governed semantic detail

Defines clinical service offerings (e.g., Cardiology Outpatient Clinic, MRI Imaging, Acute Stroke Care), specialty classifications, and clinical scope.

20.13.10.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Service Catalogue Publishing (p. 291)	Established

20.13.11. Service: Healthcare Service Directory Query

Property	Value	Meaning
Reference Id	`HCM-SV-healthcare-service-directory-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-HEALTHCARE-SERVICE-DIRECTORY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:175 (p. 1439)	Source standing does not approve the derived assertion.

20.13.11.1. Definition / governed semantic detail

Exposes published clinical service offerings.

20.13.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Service Catalogue Publishing (p. 291)	Established

20.13.12. Function: Maintain Service / Location / Provider Map

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-service-location-provider-map`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-SERVICE-LOCATION-PROVIDER-MAP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:177 (p. 1439)	Source standing does not approve the derived assertion.

20.13.12.1. Definition / governed semantic detail

Binds health service definitions to delivering organisations, physical clinic locations, and participating practitioner roles.

20.13.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Service-Location-Provider Binding (p. 291)	Established

20.13.13. Service: Service Provision Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-service-provision-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-SERVICE-PROVISION-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	em:178 (p. 1439)	Source standing does not approve the derived assertion.

20.13.13.1. Definition / governed semantic detail

Exposes multi-dimensional resolution queries answering who, where, and when healthcare services are delivered.

20.13.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Service-Location-Provider Binding (p. 291)	Established

20.13.14. Function: Maintain Service Availability

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-service-availability`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-SERVICE-AVAILABILITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:180 (p. 1439)	Source standing does not approve the derived assertion.

20.13.14.1. Definition / governed semantic detail

Governs regular operating hours, emergency intake availability, and seasonal clinic schedules.

20.13.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Operating Schedule Specification (p. 292)	Established

20.13.15. Service: Service Availability Query

Property	Value	Meaning
Reference Id	`HCM-SV-service-availability-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-SERVICE-AVAILABILITY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:181 (p. 1439)	Source standing does not approve the derived assertion.

20.13.15.1. Definition / governed semantic detail

Discloses operational intake hours and open status.

20.13.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Operating Schedule Specification (p. 292)	Established

20.13.16. Function: Maintain Service Eligibility Rules

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-service-eligibility-rules`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-SERVICE-ELIGIBILITY-RULES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	em:183 (p. 1439)	Source standing does not approve the derived assertion.

20.13.16.1. Definition / governed semantic detail

Configures catchment criteria, age thresholds, referral requirements, and clinical indications for service access.

20.13.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Eligibility & Catchment Specification (p. 293)	Established

20.13.17. Service: Service Eligibility Evaluation

Property	Value	Meaning
Reference Id	`HCM-SV-service-eligibility-evaluation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-SERVICE-ELIGIBILITY-EVALUATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:184 (p. 1439)	Source standing does not approve the derived assertion.

20.13.17.1. Definition / governed semantic detail

Evaluates referral eligibility for intake coordinators.

20.13.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Eligibility & Catchment Specification (p. 293)	Established

20.14. BEC CT2: Health Materiel Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-health-product-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.
Established name	Health Product Administration	Current display rename is Proposed; meaning unchanged.
Name history	Health Product Administration	Names do not establish another identity.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:99 (p. 1365); enabling:208 (p. 1368)	Source standing does not approve the derived assertion.

20.14.1. Capability Overview

Health Product Administration retains the reference context for medications, implants, blood products and consumables. Product definitions, formularies and batch or serial information help explain what is being referenced without assigning a supply-chain implementation to Harmonia. No Feature decomposition is established for this responsibility in the present baseline.

20.14.2. Definition — Reviewed candidate

Health Product Administration retains the reference context for medications, implants, blood products and consumables. Product definitions, formularies and batch or serial information help explain what is being referenced without assigning a supply-chain implementation to Harmonia. No Feature decomposition is established for this responsibility in the present baseline.

20.14.3. Boundary — retained v0.1 source explanation

Product representation does not perform procurement, stock control, dispensing or clinical administration. The contextual child descriptions do not establish additional canonical capabilities.

20.14.4. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.14.5. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.14.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.14.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Health Materiel Profile Administration (p. 301)	capability CT3	Proposed / Proposed
BEC CT3: Health Materiel Catalogue & Formulary Administration (p. 302)	capability CT3	Proposed / Proposed
BEC CT3: Health Materiel Traceability Administration (p. 304)	capability CT3	Proposed / Proposed

20.14.8. BEC CT3: Health Materiel Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-health-product-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION.CT3-BEC-HEALTH-PRODUCT-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Materiel Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Health Product Definition	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:210; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.14.8.1. Capability Overview

Administer identifying and descriptive information about health materiel referenced in healthcare information and activity.

20.14.8.2. Definition — Reviewed candidate

Administer identifying and descriptive information about health materiel referenced in healthcare information and activity.

20.14.8.3. Boundary

Reference representation is distinct from procurement, manufacturing and prescribing.

20.14.8.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.14.8.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:210; information-responsibility.md:10

20.14.8.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.14.8.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Classification of pharmaceutical and consumable products.

Previous overview: Medication, blood-product, implant and consumable descriptions provide reference context for care information. Product identity and classification are retained without turning this reference responsibility into procurement or prescribing.

Previous intent: Provide interpretable product context for health information without acquiring product-management authority.

20.14.8.8. Prior Candidate Derivation

v0.3 reconciliation: `BEC-1BE2ADD6` (line 4188). Working locators are not identity aliases.

20.14.9. BEC CT3: Health Materiel Catalogue & Formulary Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-product-formulary-catalogue`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION.CT3-BEC-PRODUCT-FORMULARY-CATALOGUE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Materiel Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Product Formulary & Catalogue	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:211; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.14.9.1. Capability Overview

Administer health-materiel catalogue and externally approved formulary information, preserving approval and version standing.

20.14.9.2. Definition — Reviewed candidate

Administer health-materiel catalogue and externally approved formulary information, preserving approval and version standing.

20.14.9.3. Boundary

Represents externally approved formularies; it does not acquire formulary approval or dispensing authority.

20.14.9.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.14.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:211; information-responsibility.md:10

20.14.9.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.14.9.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Management of approved facility medication formularies.

Previous overview: An approved facility formulary or product catalogue provides a qualified set of products relevant to local care. Representing its membership and source standing remains distinct from deciding formulary approval and from generic terminology governance.

Previous intent: Preserve the applicable approved product set and the source of its standing.

20.14.9.8. Prior Candidate Derivation

v0.3 reconciliation: `BEC-F3F076F6` (line 4255). Working locators are not identity aliases.

20.14.10. BEC CT3: Health Materiel Traceability Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-product-batch-serial-tracking`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION.CT3-BEC-PRODUCT-BATCH-SERIAL-TRACKING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Health Materiel Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Product Batch & Serial Tracking	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:212; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.14.10.1. Capability Overview

Administer batch/lot, serial, supplier, manufacturer, receipt/delivery and relevant movement/provenance information concerning health materiel.

20.14.10.2. Definition — Reviewed candidate

Administer batch/lot, serial, supplier, manufacturer, receipt/delivery and relevant movement/provenance information concerning health materiel.

20.14.10.3. Boundary

Materiel traceability does not make Harmonia a procurement, stock-control or supply-chain manager; specimen movement remains Clinical Logistics Coordination.

20.14.10.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.14.10.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:212; information-responsibility.md:10

20.14.10.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.14.10.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Supply chain batch and lot management.

Previous overview: Batch, lot and serial references make a product instance or supply-chain association traceable when care information needs it. This reference scope does not run inventory, recall operations or the supply chain.

Previous intent: Allow material product references in care information to be traced to their batch or instance context.

Previous boundary: Retains product trace context without running stock control or supply-chain operations.

20.14.10.8. Prior Candidate Derivation

v0.3 reconciliation: `BEC-0ACE1412` (line 4331). Working locators are not identity aliases.

20.15. BEC CT2: Clinical Device Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-device-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:103 (p. 1365); enabling:215 (p. 1368); em:193 (p. 1440)	Source standing does not approve the derived assertion.

20.15.1. Capability Overview

Clinical Device Administration represents device types, identifiable instances, patient or location associations and communication endpoints. This lets device-originated information be interpreted in relation to the actual device and care context, rather than merely a network address. Type, instance, assignment and connectivity each answer a different question about the source of device data.

20.15.2. Definition — Reviewed candidate

Clinical Device Administration represents device types, identifiable instances, patient or location associations and communication endpoints. This lets device-originated information be interpreted in relation to the actual device and care context, rather than merely a network address. Type, instance, assignment and connectivity each answer a different question about the source of device data.

20.15.3. Boundary — retained v0.1 source explanation

Device registration is not maintenance, release management or clinical interpretation. An endpoint credential does not by itself authorise clinical use of the device.

20.15.4. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

20.15.5. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.15.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.15.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Maintain Device Definition (p. 320)	function	Canonical / Established
Service: Device Definition Query (p. 320)	service	Canonical / Established
Function: Maintain Device Instance Identity (p. 321)	function	Canonical / Established
Service: Device Instance Resolution (p. 322)	service	Canonical / Established
Function: Maintain Device Association (p. 322)	function	Canonical / Established
Service: Device Association Query (p. 323)	service	Canonical / Established
Function: Maintain Device Communication Endpoints (p. 323)	function	Canonical / Established
Service: Device Endpoint Resolution (p. 324)	service	Canonical / Established
BEC CT3: Clinical Device Profile Administration (p. 307)	capability CT3	Proposed / Proposed
BEC CT3: Device Utilisation Administration (p. 311)	capability CT3	Proposed / Proposed
BEC CT3: Device OA&M Metadata Management (p. 316)	capability CT3	Proposed / Proposed
BEC CT3: Device Connectivity Compliance Management (p. 319)	capability CT3	Proposed / Proposed

20.15.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Clinical Technology Management (p. 66)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

20.15.9. BEC CT3: Clinical Device Profile Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-device-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Name history	Device Definition	Names do not establish another identity.

Property	Value	Meaning
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:218; information-responsibility.md:10	Source standing does not approve the derived assertion.

20.15.9.1. Capability Overview

Administer device type, instance, identifying and descriptive information needed to recognise clinical equipment and its stated capabilities.

20.15.9.2. Definition — Reviewed candidate

Administer device type, instance, identifying and descriptive information needed to recognise clinical equipment and its stated capabilities.

20.15.9.3. Boundary

Describes device types and capabilities without taking regulatory approval or clinical-use authority.

20.15.9.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

20.15.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:218; information-responsibility.md:10

20.15.9.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.15.9.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Specifying medical equipment types, models, and regulatory classifications.

Previous overview: A device type has a model, regulatory description and stated capabilities used to interpret equipment context. This type-level responsibility differs from identifying the actual physical instance that produced information.

Previous intent: Make the stated type and capability of a device understandable before its instance is used as an information source.

20.15.9.8. Consolidated / decomposed predecessors

BEC CT3: Clinical Device Profile Administration (p. 307); BEC CT3: Device Instance (p. 934)

20.15.9.9. Prior Candidate Derivation

v0.3 reconciliation: `BEC-EF278D25` (line 4409). Working locators are not identity aliases.

20.15.9.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Device Type Specification (p. 309)	feature FT	Canonical / Proposed
Feature: Device Serial Tracking (p. 310)	feature FT	Canonical / Proposed

20.15.9.11. Feature: Device Type Specification

Property	Value	Meaning
Reference Id	`FEAT-EM-28`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION.FEAT-EM-28`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Clinical Device Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Established broader owner	BEC CT2: Clinical Device Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:220 (p. 1368)	Source standing does not approve the derived assertion.

20.15.9.11.1. Feature Overview

Device type specification provides model-level capabilities and regulatory context against which individual devices can be understood. It describes the kind of equipment without substituting for the release, condition or deployment of a particular instance.

20.15.9.11.2. Definition / governed semantic detail

Catalog medical device models, regulatory approvals, and capabilities.

20.15.9.11.3. Proposed Placement Rationale

The named contextual responsibility provides a semantically narrower CT3 frame; the established broader capability retains responsibility through ancestry.

20.15.9.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Device Definition (p. 320)	Established
From: associated feature	Service: Device Definition Query (p. 320)	Established

20.15.9.12. Feature: Device Serial Tracking

Property	Value	Meaning
Reference Id	FEAT-EM-29	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION.FEAT-EM-29	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Clinical Device Profile Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-INSTANCE.FEAT-EM-29	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Device Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:224 (p. 1368)	Source standing does not approve the derived assertion.

20.15.9.12.1. Feature Overview

Serial tracking makes individual hardware distinguishable within the device estate. A unique device reference supports subsequent assignment and source interpretation, while remaining separate from evidence that the device is maintained, connected or fit for a particular clinical use.

20.15.9.12.2. Definition / governed semantic detail

Ingest and maintain unique device identifier (UDI) records and serial numbers.

20.15.9.12.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.15.9.12.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Device Instance Identity (p. 321)	Established
From: associated feature	Service: Device Instance Resolution (p. 322)	Established

20.15.10. BEC CT3: Device Utilisation Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-device-utilisation-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

20.15.10.1. Capability Overview

Administer qualified device-to-subject/place allocation, location, utilisation and operational readiness information, including the time at which an association applies. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

20.15.10.2. Definition — Reviewed candidate

Administer qualified device-to-subject/place allocation, location, utilisation and operational readiness information, including the time at which an association applies.

20.15.10.3. Intent

Administer qualified device-to-subject/place allocation, location, utilisation and operational readiness information, including the time at which an association applies.

20.15.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

20.15.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.15.10.6. Consolidated / decomposed predecessors

BEC CT3: Device Association (p. 935); BEC CT3: Clinical Resource Management (p. 889)

20.15.10.7. Prior Candidate Derivation

v0.3 reconciliation: `BEC-556B4A3F` (line 7681). Working locators are not identity aliases.

20.15.10.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Device-Patient/Location Assignment (p. 312)	feature FT	Canonical / Proposed
Feature: Mobile Clinical Equipment Tracking (p. 313)	feature FT	Canonical / Proposed
Function: Track Mobile Clinical Asset Operational State (p. 314)	function	Canonical / Proposed
Service: Clinical Resource Availability Query (p. 315)	service	Canonical / Proposed
Feature: Device Association Lineage (p. 315)	feature FT	Proposed / Proposed

20.15.10.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Clinical Technology Management (p. 66)	Proposed

20.15.10.10. Feature: Device-Patient/Location Assignment

Property	Value	Meaning
Reference Id	`FEAT-EM-30`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-EM-30`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Device Utilisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-ASSOCIATION.FEAT-EM-30`	Direct to this identity; never reusable.

Property	Value	Meaning
Established broader owner	BEC CT2: Clinical Device Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:228 (p. 1368)	Source standing does not approve the derived assertion.

20.15.10.10.1. Feature Overview

Dynamic device association preserves which patient or physical place a device is linked to as assignments change. This is essential context for interpreting telemetry: a stable device identity does not imply that every observation concerns the same patient over time.

20.15.10.10.2. Definition / governed semantic detail

Track dynamic association between a telemetry monitor, patient, and physical bed.

20.15.10.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.15.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Device Association (p. 322)	Established
From: associated feature	Service: Device Association Query (p. 323)	Established

20.15.10.11. Feature: Mobile Clinical Equipment Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-14`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-HSO-14`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Device Utilisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT.FEAT-HSO-14`	Direct to this identity; never reusable.
Established broader owner	BEC CT3: Clinical Resource Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:514 (p. 1377)	Source standing does not approve the derived assertion.

20.15.10.11.1. Feature Overview

Equipment tracking provides the dynamic location and allocation context of shared devices. It distinguishes an identifiable asset from one available for the present operational need, while leaving maintenance, ownership and clinical authorisation with their respective responsibilities.

20.15.10.11.2. Definition / governed semantic detail

Track physical location and allocation status of shared clinical equipment (infusion pumps, ventilators, mobile telemetry).

20.15.10.11.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.15.10.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Track Mobile Clinical Asset Operational State (p. 314)	Established
From: associated feature	Service: Clinical Resource Availability Query (p. 315)	Established

20.15.10.12. Function: Track Mobile Clinical Asset Operational State

Property	Value	Meaning
Reference Id	`HCM-FN-track-mobile-clinical-asset-operational-state`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FN-TRACK-MOBILE-CLINICAL-ASSET-OPERATIONAL-STATE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Device Utilisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT.FN-TRACK-MOBILE-CLINICAL-ASSET-OPERATIONAL-STATE`	Direct to this identity; never reusable.
Established broader owner	BEC CT3: Clinical Resource Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	hso:172 (p. 1460)	Source standing does not approve the derived assertion.

20.15.10.12.1. Definition / governed semantic detail

Manages the real-time location and operational readiness of mobile ventilators, infusion pumps, and telemetry transmitters.

20.15.10.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Mobile Clinical Equipment Tracking (p. 313)	Established

20.15.10.13. Service: Clinical Resource Availability Query

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-resource-availability-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.SV-CLINICAL-RESOURCE-AVAILABILITY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Device Utilisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT.SV-CLINICAL-RESOURCE-AVAILABILITY-QUERY`	Direct to this identity; never reusable.
Established broader owner	BEC CT3: Clinical Resource Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	hso:173 (p. 1460)	Source standing does not approve the derived assertion.

20.15.10.13.1. Definition / governed semantic detail

Exposes mobile equipment availability.

20.15.10.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Mobile Clinical Equipment Tracking (p. 313)	Established

20.15.10.14. Feature: Device Association Lineage

Property	Value	Meaning
Reference Id	`HCM-FT-device-association-lineage`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-STEP2-DEVICE-ASSOCIATION-LINEAGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Device Utilisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-ASSOCIATION.FEAT-STEP2-DEVICE-ASSOCIATION-LINEAGE`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

20.15.10.14.1. Feature Overview

A telemetry record needs to be interpreted against the association applicable when it arose, rather than the device current location alone.

20.15.10.14.2. Definition — Proposed

Preserve the attributed history and effective basis of device-to-subject/place associations.

20.15.10.14.3. Boundary

Association history does not interpret physiological observations or prove clinical correctness.

20.15.10.14.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.15.10.14.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

20.15.10.14.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-FC971E86` (line 4624). Working locators are not identity aliases.

20.15.11. BEC CT3: Device OA&M Metadata Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-device-oa-m-metadata-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-OA-M-METADATA-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

20.15.11.1. Capability Overview

Manage operational and administrative metadata needed to understand how a clinical device is connected, monitored and administered. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

20.15.11.2. Definition — Reviewed candidate

Manage operational and administrative metadata needed to understand how a clinical device is connected, monitored and administered.

20.15.11.3. Intent

Manage operational and administrative metadata needed to understand how a clinical device is connected, monitored and administered.

20.15.11.4. Boundary

Connectivity, monitoring and administration metadata do not perform device maintenance, interpret clinical telemetry or determine conformance.

20.15.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.15.11.6. Consolidated / decomposed predecessors

BEC CT3: Device Association (p. 935); BEC CT3: Device Connectivity (p. 936)

20.15.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Device Endpoint Registration (p. 318)	feature FT	Canonical / Proposed

20.15.11.8. Feature: Device Endpoint Registration

Property	Value	Meaning
Reference Id	`FEAT-EM-31`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-OA-M-METADATA-MANAGEMENT.FEAT-EM-31`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration → BEC CT3: Device OA&M Metadata Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-CONNECTIVITY.FEAT-EM-31`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Device Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:232 (p. 1368)	Source standing does not approve the derived assertion.

20.15.11.8.1. Feature Overview

Endpoint registration supplies the interfaces and credential context through which a diagnostic or telemetry feed can participate. It describes provisioned connectivity rather than proving that data has arrived, been clinically interpreted or acquired authoritative standing.

20.15.11.8.2. Definition / governed semantic detail

Register communication interfaces and security credentials for diagnostic telemetry feeds.

20.15.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

20.15.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Device Communication Endpoints (p. 323)	Established
From: associated feature	Service: Device Endpoint Resolution (p. 324)	Established

20.15.12. BEC CT3: Device Connectivity Compliance Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-device-connectivity-compliance-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-CONNECTIVITY-COMPLIANCE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

20.15.12.1. Capability Overview

Manage information about whether device connectivity satisfies applicable standards, policies and integration constraints. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

20.15.12.2. Definition — Reviewed candidate

Manage information about whether device connectivity satisfies applicable standards, policies and integration constraints.

20.15.12.3. Intent

Manage information about whether device connectivity satisfies applicable standards, policies and integration constraints.

20.15.12.4. Boundary

Endpoint registration alone does not establish compliance; conformity concerns applicable standards, policies and integration constraints.

20.15.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

20.15.13. Function: Maintain Device Definition

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-device-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:196 (p. 1440)	Source standing does not approve the derived assertion.

20.15.13.1. Definition / governed semantic detail

Models medical device models, manufacturers, regulatory classifications (e.g., TGA/FDA classes), and calibration specifications.

20.15.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Device Type Specification (p. 309)	Established

20.15.14. Service: Device Definition Query

Property	Value	Meaning
Reference Id	`HCM-SV-device-definition-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-DEFINITION-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	em:197 (p. 1440)	Source standing does not approve the derived assertion.

20.15.14.1. Definition / governed semantic detail

Discloses approved medical device models and technical parameters.

20.15.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Device Type Specification (p. 309)	Established

20.15.15. Function: Maintain Device Instance Identity

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-device-instance-identity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-INSTANCE-IDENTITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:199 (p. 1440)	Source standing does not approve the derived assertion.

20.15.15.1. Definition / governed semantic detail

Tracks physical serial numbers, Unique Device Identifiers (UDI), asset tags, and firmware versions.

20.15.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Device Serial Tracking (p. 310)	Established

20.15.16. Service: Device Instance Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-device-instance-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-INSTANCE-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:200 (p. 1440)	Source standing does not approve the derived assertion.

20.15.16.1. Definition / governed semantic detail

Resolves physical device instances from scanned barcodes or UDIs.

20.15.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Device Serial Tracking (p. 310)	Established

20.15.17. Function: Maintain Device Association

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-device-association`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-ASSOCIATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:202 (p. 1440)	Source standing does not approve the derived assertion.

20.15.17.1. Definition / governed semantic detail

Records active associations between clinical devices, patient encounters, and specific care-places (e.g., Bed 4 Vital Signs Monitor).

20.15.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Device-Patient/Location Assignment (p. 312)	Established

20.15.18. Service: Device Association Query

Property	Value	Meaning
Reference Id	`HCM-SV-device-association-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-ASSOCIATION-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:203 (p. 1440)	Source standing does not approve the derived assertion.

20.15.18.1. Definition / governed semantic detail

Discloses active device-to-patient and device-to-location bindings.

20.15.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Device-Patient/Location Assignment (p. 312)	Established

20.15.19. Function: Maintain Device Communication Endpoints

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-device-communication-endpoints`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-COMMUNICATION-ENDPOINTS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:205 (p. 1440)	Source standing does not approve the derived assertion.

20.15.19.1. Definition / governed semantic detail

Manages network telemetry addresses, gateway ports, and protocol bindings for medical devices.

20.15.19.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Device Endpoint Registration (p. 318)	Established

20.15.20. Service: Device Endpoint Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-device-endpoint-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-ENDPOINT-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	em:206 (p. 1440)	Source standing does not approve the derived assertion.

20.15.20.1. Definition / governed semantic detail

Exposes network addresses for telemetry routing.

20.15.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Device Endpoint Registration (p. 318)	Established

21. BEC CT1: Care Administration Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-administration-information-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT`	Current proposed context where marked Proposed.
Name history	Care Administration Information Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668	Source standing does not approve the derived assertion.

21.1. Capability Overview

Administer the information and progression of care access, requests, encounters, records and plans across responsible care systems.

21.2. Definition — Reviewed candidate

Administer the information and progression of care access, requests, encounters, records and plans across responsible care systems.

21.3. Boundary

Information administration does not perform the clinical act, own host booking engines or infer Transfer of Care from referral acceptance.

21.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668

21.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Receive, correlate and communicate the administrative requests, episodes, fulfilment records and documents through which care is arranged and recorded.

Previous overview: Referrals, bookings, encounters, orders and clinical documents have different progression and acknowledgement meanings. Their common ability is to make care administration intelligible across organisations without making those transactions interchangeable.

Previous intent: Keep the requested, accepted, performed and recorded aspects of care administration distinguishable as information crosses systems.

21.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Client-Centred Administration (p. 328)	capability CT2	Canonical / Proposed
BEC CT2: Referral Administration (p. 342)	capability CT2	Canonical / Proposed
BEC CT2: Scheduling Administration (p. 355)	capability CT2	Canonical / Proposed
BEC CT2: Episode & Encounter Administration (p. 363)	capability CT2	Canonical / Proposed
BEC CT2: Order Administration (p. 383)	capability CT2	Canonical / Proposed
BEC CT2: Diagnostic Administration (p. 399)	capability CT2	Canonical / Proposed
BEC CT2: Medication Administration (p. 417)	capability CT2	Canonical / Proposed
BEC CT2: Procedure Administration (p. 435)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Record Administration (p. 443)	capability CT2	Canonical / Proposed
BEC CT2: Care Plan Administration (p. 455)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Communication Administration (p. 476)	capability CT2	Canonical / Proposed
BEC CT2: Funding & Coverage Administration (p. 487)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Charging & Claims Administration (p. 489)	capability CT2	Canonical / Proposed

21.9. BEC CT2: Client-Centred Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-client-centred-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:241 (p. 1369); enabling:279 (p. 1370); sa:12 (p. 1441)	Source standing does not approve the derived assertion.

21.9.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.9.2. Capability Overview

Client-Centred Administration receives demographic and registration changes and makes verified updates available to participating clinical systems. It connects administrative events from external PAS environments with the client context used across care. The enabling responsibility concerns integration of changes, rather than independently deciding the authoritative demographic truth whenever a source sends an update.

21.9.3. Definition — Reviewed candidate

Client-Centred Administration receives demographic and registration changes and makes verified updates available to participating clinical systems. It connects administrative events from external PAS environments with the client context used across care. The enabling responsibility concerns integration of changes, rather than independently deciding the authoritative demographic truth whenever a source sends an update.

21.9.4. Boundary — retained v0.1 source explanation

Ingestion and distribution do not establish master clinical identity reconciliation or authority over the originating registration process.

21.9.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Demographic changes materially affect downstream association and continuity, so their governed propagation is active enablement rather than passive directory context.

21.9.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.9.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.9.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.9.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Screening and Immunisation Enablement (p. 329)	capability CT3	Canonical / Proposed
Function: Receive Client Demographic Change (p. 339)	function	Canonical / Established
Service: Demographic Intake Service (p. 340)	service	Canonical / Established
Function: Distribute Governed Demographic Change (p. 341)	function	Canonical / Established
Service: Demographic Update Notification (p. 342)	service	Canonical / Established
BEC CT3: Client Intake & Demographic Update (p. 336)	capability CT3	Proposed / Proposed

21.9.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Access Management (p. 27)	Proposed

21.9.11. BEC CT3: Screening and Immunisation Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-preventive-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE`	Direct to this identity; never reusable.
Established name	Preventive Care	Current display rename is Proposed; meaning unchanged.
Name history	Preventive Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:372 (p. 1372); enabling:419 (p. 1374); sd:164 (p. 1452)	Source standing does not approve the derived assertion.

21.9.11.1. Capability Overview

Preventive Care enables information flows around immunisation and screening recall. Administration records can be conveyed to national registers, while established reminders can reach designated providers. This Service Delivery context supplies bounded information enablement; it does not replace the broader Business Capability Preventative Health or its programme and cohort responsibilities.

21.9.11.2. Boundary

Submission does not perform immunisation, and a recall notice does not determine individual clinical eligibility or complete screening.

21.9.11.3. Harmonia Architectural Relevance

Register integration and recall distribution materially connect preventative activity across participants.

21.9.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.9.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.9.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.9.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Immunisation Event Submission (p. 331)	feature FT	Canonical / Proposed
Feature: Screening Recall Notification Distribution (p. 332)	feature FT	Canonical / Proposed
Function: Syndicate Immunisation Record (p. 333)	function	Canonical / Established
Service: Immunisation History Query (p. 334)	service	Canonical / Established
Function: Project Screening Eligibility Timeline (p. 334)	function	Canonical / Established
Service: Preventive Screening Status Query (p. 335)	service	Canonical / Established

21.9.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

21.9.11.9. Feature: Immunisation Event Submission

Property	Value	Meaning
Reference Id	`FEAT-SD-13`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-13`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Screening and Immunisation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-13`	Direct to this identity; never reusable.
Evidence	enabling:421 (p. 1374)	Source standing does not approve the derived assertion.

21.9.11.9.1. Feature Overview

Immunisation submission connects a source-recorded administration event with the national register context. It supports continuity and reporting without performing the immunisation or making successful transmission equivalent to every downstream registry outcome.

21.9.11.9.2. Definition / governed semantic detail

Ingest point-of-care immunisation administration records and syndicate to national immunization registers.

21.9.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Syndicate Immunisation Record (p. 333)	Established
From: associated feature	Service: Immunisation History Query (p. 334)	Established

21.9.11.10. Feature: Screening Recall Notification Distribution

Property	Value	Meaning
Reference Id	`FEAT-SD-14`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-14`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Screening and Immunisation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-14`	Direct to this identity; never reusable.
Evidence	enabling:422 (p. 1374)	Source standing does not approve the derived assertion.

21.9.11.10.1. Feature Overview

Screening recall distribution conveys established reminders to the designated care providers. Its notification responsibility is expressible through approved event and workflow composition; the catalogue does not invent a specialised Function or transfer the clinical decision to recall the patient.

21.9.11.10.2. Definition / governed semantic detail

Distribute clinical screening reminders and recall notices to designated healthcare providers.

21.9.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Project Screening Eligibility Timeline (p. 334)	Unresolved association
From: associated feature	Service: Preventive Screening Status Query (p. 335)	Unresolved association

21.9.11.11. Function: Syndicate Immunisation Record

Property	Value	Meaning
Reference Id	`HCM-FN-syndicate-immunisation-record`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FN-SYNDICATE-IMMUNISATION-RECORD`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Screening and Immunisation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FN-SYNDICATE-IMMUNISATION-RECORD`	Direct to this identity; never reusable.
Evidence	sd:168 (p. 1452)	Source standing does not approve the derived assertion.

21.9.11.11.1. Definition / governed semantic detail

Records vaccine encounters and syndicates records to national immunisation registers (e.g., AIR).

21.9.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Immunisation Event Submission (p. 331)	Established

21.9.11.12. Service: Immunisation History Query

Property	Value	Meaning
Reference Id	`HCM-SV-immunisation-history-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.SV-IMMUNISATION-HISTORY-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Screening and Immunisation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.SV-IMMUNISATION-HISTORY-QUERY`	Direct to this identity; never reusable.
Evidence	sd:169 (p. 1452)	Source standing does not approve the derived assertion.

21.9.11.12.1. Definition / governed semantic detail

Discloses complete vaccination history.

21.9.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Immunisation Event Submission (p. 331)	Established

21.9.11.13. Function: Project Screening Eligibility Timeline

Property	Value	Meaning
Reference Id	`HCM-FN-project-screening-eligibility-timeline`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FN-PROJECT-SCREENING-ELIGIBILITY-TIMELINE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Screening and Immunisation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FN-PROJECT-SCREENING-ELIGIBILITY-TIMELINE`	Direct to this identity; never reusable.
Evidence	sd:172 (p. 1452); sd:171 (p. 1452)	Source standing does not approve the derived assertion.

21.9.11.13.1. Definition / governed semantic detail

Tracks age-based and risk-based screening schedules across primary care.

Feature association not established: `FEAT-SD-14 — Screening Recall Notification Distribution` is adjudicated sufficient through reusable composition/workflow above. Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.

21.9.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Screening Recall Notification Distribution (p. 332)	Unresolved association

21.9.11.14. Service: Preventive Screening Status Query

Property	Value	Meaning
Reference Id	`HCM-SV-preventive-screening-status-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.SV-PREVENTIVE-SCREENING-STATUS-QUERY`	Current proposed context where marked Proposed.

Property	Value	Meaning
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Screening and Immunisation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.SV-PREVENTIVE-SCREENING-STATUS-QUERY`	Direct to this identity; never reusable.
Evidence	sd:173 (p. 1452); sd:171 (p. 1452)	Source standing does not approve the derived assertion.

21.9.11.14.1. Definition / governed semantic detail

Discloses screening due dates to primary care providers.

Feature association not established: `FEAT-SD-14 — Screening Recall Notification Distribution` is adjudicated sufficient through reusable composition/workflow above. Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.

21.9.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Screening Recall Notification Distribution (p. 332)	Unresolved association

21.9.12. BEC CT3: Client Intake & Demographic Update

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-client-intake-demographic-update`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.9.12.1. Capability Overview

Administer the established client intake & demographic update information and coordination responsibility while preserving its source authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.9.12.2. Definition — Proposed

Administer the established client intake & demographic update information and coordination responsibility while preserving its source authority.

21.9.12.3. Intent

Administer the established client intake & demographic update information and coordination responsibility while preserving its source authority.

21.9.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.9.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.9.12.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-25DB15BA` (line 4745). Working locators are not identity aliases.

21.9.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Demographic Update Ingestion (p. 337)	feature FT	Canonical / Proposed
Feature: Demographic Change Distribution (p. 338)	feature FT	Canonical / Proposed

21.9.12.8. Feature: Demographic Update Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-01`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE.FEAT-SA-01`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Client Intake & Demographic Update	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FEAT-SA-01`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Client-Centred Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:281 (p. 1370)	Source standing does not approve the derived assertion.

21.9.12.8.1. Feature Overview

Demographic ingestion receives administrative changes from upstream admission and registration systems. It makes externally established updates available for governed processing without treating each incoming assertion as an independent master-person adjudication by Harmonia.

21.9.12.8.2. Definition / governed semantic detail

Ingest and validate patient admission and registration updates from external PAS systems.

21.9.12.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.9.12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Client Demographic Change (p. 339)	Established
From: associated feature	Service: Demographic Intake Service (p. 340)	Established

21.9.12.9. Feature: Demographic Change Distribution

Property	Value	Meaning
Reference Id	`FEAT-SA-02`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE.FEAT-SA-02`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration → BEC CT3: Client Intake & Demographic Update	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FEAT-SA-02`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Client-Centred Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:282 (p. 1370)	Source standing does not approve the derived assertion.

21.9.12.9.1. Feature Overview

Change distribution propagates verified demographic updates to participating clinical systems. It supports consistent subject context across the network while keeping delivery to each destination distinguishable from that destination's eventual incorporation or business response.

21.9.12.9.2. Definition / governed semantic detail

Fan out verified client demographic updates to downstream clinical subsystems.

21.9.12.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.9.12.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Distribute Governed Demographic Change (p. 341)	Established
From: associated feature	Service: Demographic Update Notification (p. 342)	Established

21.9.13. Function: Receive Client Demographic Change

Property	Value	Meaning
Reference Id	`HCM-FN-receive-client-demographic-change`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-RECEIVE-CLIENT-DEMOGRAPHIC-CHANGE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-RECEIVE-CLIENT-DEMOGRAPHIC-CHANGE`	Direct to this identity; never reusable.
Evidence	sa:18 (p. 1441)	Source standing does not approve the derived assertion.

21.9.13.1. Definition / governed semantic detail

Ingests administrative demographic updates from PAS/registration feeds.

21.9.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Demographic Update Ingestion (p. 337)	Established

21.9.14. Service: Demographic Intake Service

Property	Value	Meaning
Reference Id	`HCM-SV-demographic-intake-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-INTAKE-SERVICE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-INTAKE-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:19 (p. 1441)	Source standing does not approve the derived assertion.

21.9.14.1. Definition / governed semantic detail

Exposed to departmental registration systems.

21.9.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Demographic Update Ingestion (p. 337)	Established

21.9.15. Function: Distribute Governed Demographic Change

Property	Value	Meaning
Reference Id	`HCM-FN-distribute-governed-demographic-change`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-DISTRIBUTE-GOVERNED-DEMOGRAPHIC-CHANGE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-DISTRIBUTE-GOVERNED-DEMOGRAPHIC-CHANGE`	Direct to this identity; never reusable.
Evidence	sa:21 (p. 1441)	Source standing does not approve the derived assertion.

21.9.15.1. Definition / governed semantic detail

Propagates verified demographic modifications to subscribed clinical subsystems.

21.9.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Demographic Change Distribution (p. 338)	Established

21.9.16. Service: Demographic Update Notification

Property	Value	Meaning
Reference Id	`HCM-SV-demographic-update-notification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-UPDATE-NOTIFICATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Client-Centred Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Client-Centred Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-UPDATE-NOTIFICATION`	Direct to this identity; never reusable.
Evidence	sa:22 (p. 1441)	Source standing does not approve the derived assertion.

21.9.16.1. Definition / governed semantic detail

Emits change notifications across the HIE.

21.9.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Demographic Change Distribution (p. 338)	Established

21.10. BEC CT2: Referral Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-referral-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:242 (p. 1369); enabling:284 (p. 1370); sa:28 (p. 1441)	Source standing does not approve the derived assertion.

21.10.1. Capability Overview

Referral Administration receives referral material, associates the supporting clinical context and tracks the recipient's explicit disposition and outcome. Its purpose is to keep the request and its progression intelligible across care boundaries. It distinguishes a request for another participant's involvement from an order for bounded fulfilment and from actual acceptance of care responsibility.

21.10.2. Definition — Reviewed candidate

Referral Administration receives referral material, associates the supporting clinical context and tracks the recipient's explicit disposition and outcome. Its purpose is to keep the request and its progression intelligible across care boundaries. It distinguishes a request for another participant's involvement from an order for bounded fulfilment and from actual acceptance of care responsibility.

21.10.3. Boundary — retained v0.1 source explanation

Transport acceptance does not establish clinical acceptance. A referral need not universally produce an appointment, encounter, order or transfer of care.

21.10.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Harmonia materially connects the referral, supporting information and reported disposition while the receiving service retains clinical and responsibility decisions.

21.10.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.10.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.10.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.10.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Receive Referral (p. 349)	function	Canonical / Established
Service: Referral Submission Service (p. 350)	service	Canonical / Established
Function: Assemble Referral Context (p. 351)	function	Canonical / Established
Service: Referral Context Query (p. 352)	service	Canonical / Established
Function: Manage Referral Progression (p. 352)	function	Canonical / Established
Service: Referral Status & Outcome Service (p. 353)	service	Canonical / Established
Process: Referral Progression Process (p. 354)	process	Canonical / Established
BEC CT3: Referral Intake & Progression (p. 344)	capability CT3	Proposed / Proposed

21.10.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Access Management (p. 27)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed

21.10.10. BEC CT3: Referral Intake & Progression

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-referral-intake-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.10.10.1. Capability Overview

Receive and correlate referral requests, supporting information, destinations, disposition and progression through responsible care systems. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.10.10.2. Definition — Reviewed candidate

Receive and correlate referral requests, supporting information, destinations, disposition and progression through responsible care systems.

21.10.10.3. Intent

Receive and correlate referral requests, supporting information, destinations, disposition and progression through responsible care systems.

21.10.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.10.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.10.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-CD3F820D` (line 4835). Working locators are not identity aliases.

21.10.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Referral Document Ingestion (p. 346)	feature FT	Canonical / Proposed
Feature: Referral Supporting Information (p. 346)	feature FT	Canonical / Proposed
Feature: Referral Status & Outcome Tracking (p. 347)	feature FT	Canonical / Proposed
Feature: Referral Destination Coordination (p. 348)	feature FT	Proposed / Proposed

21.10.10.8. Feature: Referral Document Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-03`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-03`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration → BEC CT3: Referral Intake & Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-SA-03`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Referral Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:286 (p. 1370)	Source standing does not approve the derived assertion.

21.10.10.8.1. Feature Overview

Referral ingestion establishes the received referral material within the integration boundary. It preserves the request for another participant's involvement without treating successful receipt as clinical acceptance, responsibility assumption or a promise that care will be delivered.

21.10.10.8.2. Definition / governed semantic detail

Ingest structured clinical referral messages across integration boundaries.

21.10.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.10.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Referral (p. 349)	Established
From: associated feature	Service: Referral Submission Service (p. 350)	Established

21.10.10.9. Feature: Referral Supporting Information

Property	Value	Meaning
Reference Id	`FEAT-SA-04`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-04`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration → BEC CT3: Referral Intake & Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-SA-04`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Referral Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:287 (p. 1370)	Source standing does not approve the derived assertion.

21.10.10.9.1. Feature Overview

Supporting information gives the referral its relevant history and clinical context. Association makes the material available for the recipient's consideration while preserving source provenance and the distinction between evidence supplied and the recipient's own clinical decision.

21.10.10.9.2. Definition / governed semantic detail

Collate and associate supporting clinical documentation, diagnostic history, and patient context with referral requests.

21.10.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.10.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Assemble Referral Context (p. 351)	Established
From: associated feature	Service: Referral Context Query (p. 352)	Established

21.10.10.10. Feature: Referral Status & Outcome Tracking

Property	Value	Meaning
Reference Id	`FEAT-SA-05`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-05`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration → BEC CT3: Referral Intake & Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-SA-05`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Referral Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:288 (p. 1370)	Source standing does not approve the derived assertion.

21.10.10.10.1. Feature Overview

Status and outcome tracking makes explicit referral dispositions visible to the referring participant. Acceptance, decline and redirection remain meaningful alternatives; a transport acknowledgement does not fill in a missing business decision or establish a universal referral progression.

21.10.10.10.2. Definition / governed semantic detail

Track referral operational progression and communicate referral outcomes (acceptance, decline, or redirection) to referring providers.

21.10.10.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.10.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Referral Progression (p. 352)	Established
From: associated feature	Service: Referral Status & Outcome Service (p. 353)	Established

21.10.10.11. Feature: Referral Destination Coordination

Property	Value	Meaning
Reference Id	`HCM-FT-referral-destination-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-STEP2-REFERRAL-DESTINATION-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration → BEC CT3: Referral Intake & Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-STEP2-REFERRAL-DESTINATION-COORDINATION`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

21.10.10.11.1. Feature Overview

Referral intake and status capture leave a distinct routing responsibility: finding the intended service and preserving redirection without turning technical delivery into assumed care responsibility.

21.10.10.11.2. Definition — Proposed

Coordinate qualified referral destination resolution and convey the request under its applicable disposition contract.

21.10.10.11.3. Boundary

Does not make clinical triage decisions or infer Transfer of Care from acceptance.

21.10.10.11.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.10.10.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

21.10.10.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-A551E3F4` (line 4899). Working locators are not identity aliases.

21.10.11. Function: Receive Referral

Property	Value	Meaning
Reference Id	`HCM-FN-receive-referral`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-RECEIVE-REFERRAL`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FN-RECEIVE-REFERRAL`	Direct to this identity; never reusable.
Evidence	sa:33 (p. 1441)	Source standing does not approve the derived assertion.

21.10.11.1. Definition / governed semantic detail

Validates and ingests structured electronic referrals, clinical indications, and priority categories.

21.10.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Referral Document Ingestion (p. 346)	Established

21.10.12. Service: Referral Submission Service

Property	Value	Meaning
Reference Id	`HCM-SV-referral-submission-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-SUBMISSION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-SUBMISSION-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:34 (p. 1441)	Source standing does not approve the derived assertion.

21.10.12.1. Definition / governed semantic detail

Exposed to external GP practices and referring health facilities.

21.10.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Referral Document Ingestion (p. 346)	Established

21.10.13. Function: Assemble Referral Context

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-referral-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-ASSEMBLE-REFERRAL-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FN-ASSEMBLE-REFERRAL-CONTEXT`	Direct to this identity; never reusable.
Evidence	sa:36 (p. 1441)	Source standing does not approve the derived assertion.

21.10.13.1. Definition / governed semantic detail

Gathers relevant clinical history, medications, and diagnostic investigations into the referral package.

21.10.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Referral Supporting Information (p. 346)	Established

21.10.14. Service: Referral Context Query

Property	Value	Meaning
Reference Id	`HCM-SV-referral-context-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-CONTEXT-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-CONTEXT-QUERY`	Direct to this identity; never reusable.
Evidence	sa:37 (p. 1441)	Source standing does not approve the derived assertion.

21.10.14.1. Definition / governed semantic detail

Provides comprehensive referral dossiers to intake triage clinicians.

21.10.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Referral Supporting Information (p. 346)	Established

21.10.15. Function: Manage Referral Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-referral-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-MANAGE-REFERRAL-PROGRESSION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FN-MANAGE-REFERRAL-PROGRESSION`	Direct to this identity; never reusable.
Evidence	sa:39 (p. 1441); sa:42 (p. 1441)	Source standing does not approve the derived assertion.

21.10.15.1. Definition / governed semantic detail

Tracks referral disposition in an **illustrative specialist pathway; dispositions vary**: *Submitted → Triaged → Accepted/Waitlisted → Scheduled → Discharged*.

21.10.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Referral Status & Outcome Tracking (p. 347)	Established
To: governed process	Process: Referral Progression Process (p. 354)	Established

21.10.16. Service: Referral Status & Outcome Service

Property	Value	Meaning
Reference Id	`HCM-SV-referral-status-outcome-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-STATUS-OUTCOME-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-STATUS-OUTCOME-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:40 (p. 1441); sa:42 (p. 1441)	Source standing does not approve the derived assertion.

21.10.16.1. Definition / governed semantic detail

Exposes progression milestones to referrers and patients.

21.10.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Referral Status & Outcome Tracking (p. 347)	Established

21.10.17. Process: Referral Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-referral-progression-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.PR-REFERRAL-PROGRESSION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Referral Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.PR-REFERRAL-PROGRESSION-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:102 (p. 1416); processes:103 (p. 1416)	Source standing does not approve the derived assertion.

21.10.17.1. Process Overview

This Process illustrates one specialist pathway through validation, triage and booking. It makes that pathway's progression visible without imposing it on every referral: rejection, decline or redirection can be legitimate dispositions, and a request need not universally lead to attendance. Neither a transmitted referral nor the local compound acceptance/waitlist checkpoint establishes actual responsibility transfer.

21.10.17.2. Definition / governed semantic detail

- **Owning Capability**: `ReferralAdministration`
- **Process Purpose**: Governs the end-to-end operational progression of an incoming clinical referral from receipt through specialist clinical triage, booking, and final service acceptance.
- **Scope qualification**: This illustrates a specialist booking/attendance pathway; Referral does not universally require acceptance, scheduling, attendance or discharge, or establish delivery, responsibility actually assumed or Transfer of Care. Decline, rejection and redirection remain valid dispositions without invented transition paths. `Accepted/Waitlisted` is a local compound checkpoint, not universal equivalence between acceptance and waitlisting.
- **State Progression Lifecycle**: Submitted → Intake Validated → Clinically Triaged → Accepted / Waitlisted → Scheduled → Consultation Attended → Discharged / Rejected (Illustrative specialist pathway; dispositions vary)
- **Key State Dispositions**:

- `Submitted`: Electronic referral received from GP or external facility.
- `IntakeValidated`: Administrative validation of patient details, mandatory fields, and tests.
- `ClinicallyTriaged`: Senior clinician assigns urgency category (e.g., Cat 1 urgent within 30 days).
- `Accepted/Waitlisted`: Referral accepted and placed onto the specialty waitlist.
- `Scheduled`: Outpatient appointment allocated and patient notified.
- `ConsultationAttended`: Patient seen; initial specialist assessment completed.
- `Discharged/Rejected`: Referral completed and discharged back to primary care, or formally rejected with clinical rationale.

21.10.17.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Referral Progression (p. 352)	Established

21.11. BEC CT2: Scheduling Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-scheduling-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:243 (p. 1369); enabling:290 (p. 1370); sa:48 (p. 1442)	Source standing does not approve the derived assertion.

21.11.1. Capability Overview

Scheduling Administration integrates appointment information established by departmental scheduling systems. It receives scheduled events and propagates cancellation, rescheduling and attendance changes so that care-coordination contexts do not diverge. Its concern is the coherence of scheduling information across participants rather than originating every booking decision.

21.11.2. Definition — Reviewed candidate

Scheduling Administration integrates appointment information established by departmental scheduling systems. It receives scheduled events and propagates cancellation, rescheduling and attendance changes so that care-coordination contexts do not diverge. Its concern is the coherence of scheduling information across participants rather than originating every booking decision.

21.11.3. Boundary — retained v0.1 source explanation

A synchronised booking does not prove attendance, service capacity or delivered clinical care.

21.11.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Timely appointment changes materially support coordination across systems without giving Harmonia the scheduler's clinical or departmental authority.

21.11.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.11.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.11.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.11.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Receive Appointment Notification (p. 360)	function	Canonical / Established
Service: Appointment Notification Ingress (p. 360)	service	Canonical / Established
Function: Synchronise Appointment Status (p. 361)	function	Canonical / Established
Service: Appointment Schedule Query (p. 362)	service	Canonical / Established
BEC CT3: Appointment & Booking Administration (p. 357)	capability CT3	Proposed / Proposed

21.11.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Access Management (p. 27)	Proposed

21.11.10. BEC CT3: Appointment & Booking Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-appointment-booking-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.11.10.1. Capability Overview

Administer externally supplied appointment and booking information and communicate changes in their scheduling status. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.11.10.2. Definition — Reviewed candidate

Administer externally supplied appointment and booking information and communicate changes in their scheduling status.

21.11.10.3. Intent

Administer externally supplied appointment and booking information and communicate changes in their scheduling status.

21.11.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.11.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.11.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-8FEF95D4` (line 4946). Working locators are not identity aliases.

21.11.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Appointment Notification Ingestion (p. 358)	feature FT	Canonical / Proposed
Feature: Appointment Status Synchronization (p. 359)	feature FT	Canonical / Proposed

21.11.10.8. Feature: Appointment Notification Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-06`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION.FEAT-SA-06`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration → BEC CT3: Appointment & Booking Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FEAT-SA-06`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Scheduling Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:292 (p. 1370)	Source standing does not approve the derived assertion.

21.11.10.8.1. Feature Overview

Appointment ingestion receives scheduling facts established by departmental systems. It supports shared awareness of planned care opportunities without claiming authority over the underlying booking decision or proving that attendance and clinical fulfilment will occur.

21.11.10.8.2. Definition / governed semantic detail

Ingest scheduled appointment events from departmental scheduling systems.

21.11.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.11.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Appointment Notification (p. 360)	Established
From: associated feature	Service: Appointment Notification Ingress (p. 360)	Established

21.11.10.9. Feature: Appointment Status Synchronization

Property	Value	Meaning
Reference Id	`FEAT-SA-07`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION.FEAT-SA-07`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration → BEC CT3: Appointment & Booking Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FEAT-SA-07`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Scheduling Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:293 (p. 1370)	Source standing does not approve the derived assertion.

21.11.10.9.1. Feature Overview

Status synchronisation keeps cancellation, rescheduling and attendance information coherent across coordination contexts. The purpose is propagation of the externally established booking change rather than independent scheduling or inference of a clinical outcome from appointment state.

21.11.10.9.2. Definition / governed semantic detail

Propagate booking cancellations, reschedules, and attendances across care coordination systems.

21.11.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.11.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Synchronise Appointment Status (p. 361)	Established
From: associated feature	Service: Appointment Schedule Query (p. 362)	Established

21.11.11. Function: Receive Appointment Notification

Property	Value	Meaning
Reference Id	`HCM-FN-receive-appointment-notification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.FN-RECEIVE-APPOINTMENT-NOTIFICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FN-RECEIVE-APPOINTMENT-NOTIFICATION`	Direct to this identity; never reusable.
Evidence	sa:53 (p. 1442)	Source standing does not approve the derived assertion.

21.11.11.1. Definition / governed semantic detail

Ingests appointment creation, rescheduling, cancellation, and check-in events from host PAS engines.

21.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Appointment Notification Ingestion (p. 358)	Established

21.11.12. Service: Appointment Notification Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-appointment-notification-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-NOTIFICATION-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-NOTIFICATION-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:54 (p. 1442)	Source standing does not approve the derived assertion.

21.11.12.1. Definition / governed semantic detail

Ingests booking events from departmental schedulers.

21.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Appointment Notification Ingestion (p. 358)	Established

21.11.13. Function: Synchronise Appointment Status

Property	Value	Meaning
Reference Id	`HCM-FN-synchronise-appointment-status`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.FN-SYNCHRONISE-APPOINTMENT-STATUS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FN-SYNCHRONISE-APPOINTMENT-STATUS`	Direct to this identity; never reusable.
Evidence	sa:56 (p. 1442)	Source standing does not approve the derived assertion.

21.11.13.1. Definition / governed semantic detail

Projects synchronised appointment timelines to clinical dashboards and care coordinators.

21.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Appointment Status Synchronization (p. 359)	Established

21.11.14. Service: Appointment Schedule Query

Property	Value	Meaning
Reference Id	`HCM-SV-appointment-schedule-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-SCHEDULE-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Scheduling Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-SCHEDULE-QUERY`	Direct to this identity; never reusable.
Evidence	sa:57 (p. 1442)	Source standing does not approve the derived assertion.

21.11.14.1. Definition / governed semantic detail

Exposes consolidated patient appointment schedules across facilities.

21.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Appointment Status Synchronization (p. 359)	Established

21.12. BEC CT2: Episode & Encounter Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-episode-encounter-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:244 (p. 1369); enabling:295 (p. 1370); sa:63 (p. 1442)	Source standing does not approve the derived assertion.

21.12.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.12.2. Capability Overview

This capability maintains identifiable encounter contexts and their governed progression, including association with care teams and locations. It enables information from an admission, visit or community contact to be understood within the relevant episode of activity. Encounter progression is an administrative and operational account, separate from the clinical acts that occur during it.

21.12.3. Definition — Reviewed candidate

This capability maintains identifiable encounter contexts and their governed progression, including association with care teams and locations. It enables information from an admission, visit or community contact to be understood within the relevant episode of activity. Encounter progression is an administrative and operational account, separate from the clinical acts that occur during it.

21.12.4. Boundary — retained v0.1 source explanation

The source lists ON_LEAVE in the Feature description but does not fully connect it in the Process illustration. That transition gap remains unresolved rather than completed by convention.

21.12.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Encounter context and tracked state are substantive platform responsibilities supporting correlation across records, orders and operational events.

21.12.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.12.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.12.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.12.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	capability CT3	Canonical / Proposed
Function: Establish Encounter Context (p. 377)	function	Canonical / Established
Service: Encounter Context Resolution (p. 378)	service	Canonical / Established
Function: Progress Encounter State (p. 378)	function	Canonical / Established
Service: Encounter Lifecycle Event Notification (p. 379)	service	Canonical / Established
Function: Maintain Encounter Care-Place Association (p. 380)	function	Canonical / Established
Service: Encounter Location History Query (p. 381)	service	Canonical / Established
Process: Encounter Lifecycle Process (p. 381)	process	Canonical / Established
BEC CT3: Encounter Lifecycle Management (p. 371)	capability CT3	Proposed / Proposed

21.12.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed

21.12.11. BEC CT3: Acute Care Monitoring & Event Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-acute-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE`	Direct to this identity; never reusable.
Established name	Acute Care	Current display rename is Proposed; meaning unchanged.
Name history	Acute Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:363 (p. 1372); enabling:394 (p. 1373); sd:71 (p. 1450)	Source standing does not approve the derived assertion.

21.12.11.1. Capability Overview

Acute Care provides the information context needed around acute clinical events, including externally reported deterioration and timely access to history and active medications. It helps current events be interpreted alongside prior information rather than as isolated messages. Its Service Delivery context does not become a child of Individual Care Delivery's setting-independent Business Capability hierarchy.

21.12.11.2. Boundary

Capturing a deterioration event does not determine clinical significance or prescribe an acute response.

21.12.11.3. Harmonia Architectural Relevance

Harmonia materially supplies and correlates acute-care information while clinical decisions and intervention remain with the care service.

21.12.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.12.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.12.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.12.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Acute Clinical State Event Capture (p. 366)	feature FT	Canonical / Proposed
Feature: Acute Care Clinical Context Provision (p. 367)	feature FT	Canonical / Proposed
Function: Assemble Acute Clinical Baseline (p. 368)	function	Canonical / Established
Service: Acute Admission Dossier Query (p. 369)	service	Canonical / Established
Function: Route Acute Critical Result Alert (p. 370)	function	Canonical / Established
Service: Acute Clinical Alert Dispatch (p. 370)	service	Canonical / Established

21.12.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

21.12.11.9. Feature: Acute Clinical State Event Capture

Property	Value	Meaning
Reference Id	`FEAT-SD-03`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-03`	Current proposed context where marked Proposed.

Property	Value	Meaning
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Acute Care Monitoring & Event Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-03`	Direct to this identity; never reusable.
Evidence	enabling:396 (p. 1373)	Source standing does not approve the derived assertion.

21.12.11.9.1. Feature Overview

Acute-state capture records deterioration notifications and current clinical status as externally established information. Approved reusable clinical-information composition is sufficient for this responsibility; routing a critical-result alert does not become a specialised capture Function merely because both concern urgent care.

21.12.11.9.2. Definition / governed semantic detail

Ingest clinical deterioration notifications and acute clinical status updates.

21.12.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Route Acute Critical Result Alert (p. 370)	Unresolved association
From: associated feature	Service: Acute Clinical Alert Dispatch (p. 370)	Unresolved association

21.12.11.10. Feature: Acute Care Clinical Context Provision

Property	Value	Meaning
Reference Id	`FEAT-SD-04`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-04`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Acute Care Monitoring & Event Coordination	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-04`	Direct to this identity; never reusable.
Evidence	enabling:397 (p. 1373)	Source standing does not approve the derived assertion.

21.12.11.10.1. Feature Overview

Acute context provision brings relevant history and active medications together for clinicians facing a current presentation. It supports rapid understanding without substituting the consolidated view for clinical assessment, triage or a decision about treatment.

21.12.11.10.2. Definition / governed semantic detail

Provide consolidated longitudinal history and active medication lists to acute care clinicians at point of triage.

21.12.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Assemble Acute Clinical Baseline (p. 368)	Established
From: associated feature	Service: Acute Admission Dossier Query (p. 369)	Established

21.12.11.11. Function: Assemble Acute Clinical Baseline

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-acute-clinical-baseline`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FN-ASSEMBLE-ACUTE-CLINICAL-BASELINE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Acute Care Monitoring & Event Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FN-ASSEMBLE-ACUTE-CLINICAL-BASELINE`	Direct to this identity; never reusable.
Evidence	sd:75 (p. 1450)	Source standing does not approve the derived assertion.

21.12.11.11.1. Definition / governed semantic detail

Rapidly aggregates previous discharge summaries, active medications, allergies, and diagnostic alerts upon acute hospital presentation.

21.12.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Acute Care Clinical Context Provision (p. 367)	Established

21.12.11.12. Service: Acute Admission Dossier Query

Property	Value	Meaning
Reference Id	`HCM-SV-acute-admission-dossier-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-ADMISSION-DOSSIER-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Acute Care Monitoring & Event Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-ADMISSION-DOSSIER-QUERY`	Direct to this identity; never reusable.
Evidence	sd:76 (p. 1450)	Source standing does not approve the derived assertion.

21.12.11.12.1. Definition / governed semantic detail

Delivers a high-priority clinical briefing to admitting teams.

21.12.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Acute Care Clinical Context Provision (p. 367)	Established

21.12.11.13. Function: Route Acute Critical Result Alert

Property	Value	Meaning
Reference Id	`HCM-FN-route-acute-critical-result-alert`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FN-ROUTE-ACUTE-CRITICAL-RESULT-ALERT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Acute Care Monitoring & Event Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FN-ROUTE-ACUTE-CRITICAL-RESULT-ALERT`	Direct to this identity; never reusable.
Evidence	sd:79 (p. 1450); sd:78 (p. 1450)	Source standing does not approve the derived assertion.

21.12.11.13.1. Definition / governed semantic detail

Immediately directs urgent lab/radiology alerts (e.g., critical troponin, intracranial bleed) to active acute attending clinicians.

Feature association not established: `FEAT-SD-03 — Acute Clinical State Event Capture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.

21.12.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Acute Clinical State Event Capture (p. 366)	Unresolved association

21.12.11.14. Service: Acute Clinical Alert Dispatch

Property	Value	Meaning
Reference Id	`HCM-SV-acute-clinical-alert-dispatch`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-CLINICAL-ALERT-DISPATCH`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Acute Care Monitoring & Event Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-CLINICAL-ALERT-DISPATCH`	Direct to this identity; never reusable.
Evidence	sd:80 (p. 1450); sd:78 (p. 1450)	Source standing does not approve the derived assertion.

21.12.11.14.1. Definition / governed semantic detail

Dispatches urgent interruptive alerts to on-duty ward teams.

Feature association not established: `FEAT-SD-03 — Acute Clinical State Event Capture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.

21.12.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Acute Clinical State Event Capture (p. 366)	Unresolved association

21.12.12. BEC CT3: Encounter Lifecycle Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-encounter-lifecycle-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.12.12.1. Capability Overview

Administer the established encounter lifecycle management information and coordination responsibility while preserving its source authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.12.12.2. Definition — Proposed

Administer the established encounter lifecycle management information and coordination responsibility while preserving its source authority.

21.12.12.3. Intent

Administer the established encounter lifecycle management information and coordination responsibility while preserving its source authority.

21.12.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.12.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.12.12.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-1C528FC1` (line 5044). Working locators are not identity aliases.

21.12.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Encounter Context Creation (p. 373)	feature FT	Canonical / Proposed
Feature: Encounter State Progression (p. 374)	feature FT	Canonical / Proposed
Feature: Encounter Bed/Location Association (p. 375)	feature FT	Canonical / Proposed
Feature: Encounter History Retrieval (p. 376)	feature FT	Proposed / Proposed

21.12.12.8. Feature: Encounter Context Creation

Property	Value	Meaning
Reference Id	`FEAT-SA-08`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-08`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Encounter Lifecycle Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-SA-08`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Episode & Encounter Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:297 (p. 1370)	Source standing does not approve the derived assertion.

21.12.12.8.1. Feature Overview

Encounter context creation supplies an identifiable frame for information concerning a particular admission, visit or contact. Its governed tracking authority concerns the encounter representation within Harmonia; it does not make Harmonia the originator of every clinical fact associated with that encounter.

21.12.12.8.2. Definition / governed semantic detail

Create and maintain authoritative encounter tracking contexts for hospital admissions, outpatient visits, and community contacts.

21.12.12.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.12.12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Establish Encounter Context (p. 377)	Established
From: associated feature	Service: Encounter Context Resolution (p. 378)	Established

21.12.12.9. Feature: Encounter State Progression

Property	Value	Meaning
Reference Id	`FEAT-SA-09`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-09`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Encounter Lifecycle Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-SA-09`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Episode & Encounter Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:298 (p. 1370)	Source standing does not approve the derived assertion.

21.12.12.9.1. Feature Overview

Encounter progression preserves the difference between planned, arrived, active, leave and discharged circumstances. These states make administrative activity interpretable without implying a clinical outcome. The source's ON_LEAVE transition detail remains incomplete in the corresponding Process and is not repaired here.

21.12.12.9.2. Definition / governed semantic detail

Track encounter status transitions (`PLANNED` → `ARRIVED` → `IN_PROGRESS` → `ON_LEAVE` → `DISCHARGED`).

21.12.12.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.12.12.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Progress Encounter State (p. 378)	Established
From: associated feature	Service: Encounter Lifecycle Event Notification (p. 379)	Established

21.12.12.10. Feature: Encounter Bed/Location Association

Property	Value	Meaning
Reference Id	`FEAT-SA-10`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-10`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Encounter Lifecycle Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-SA-10`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Episode & Encounter Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:299 (p. 1370)	Source standing does not approve the derived assertion.

21.12.12.10.1. Feature Overview

Encounter association keeps the relevant place and care-team context aligned with the episode as the patient moves. Location changes are meaningful operational facts, but they do not themselves establish a transfer of clinical responsibility.

21.12.12.10.2. Definition / governed semantic detail

Dynamically maintain the association between an encounter, care team, and physical bed or ward location.

21.12.12.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.12.12.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Encounter Care-Place Association (p. 380)	Established
From: associated feature	Service: Encounter Location History Query (p. 381)	Established

21.12.12.11. Feature: Encounter History Retrieval

Property	Value	Meaning
Reference Id	`HCM-FT-encounter-history-retrieval`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-STEP2-ENCOUNTER-HISTORY-RETRIEVAL`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration → BEC CT3: Encounter Lifecycle Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-STEP2-ENCOUNTER-HISTORY-RETRIEVAL`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

21.12.12.11.1. Feature Overview

Consumers need to interpret prior care-place and episode relationships, not just the latest encounter state; historical retrieval remains bounded by source attribution and access policy.

21.12.12.11.2. Definition — Proposed

Expose qualified episode, movement and encounter-history context.

21.12.12.11.3. Boundary

A historical view does not retrospectively alter clinical events or acquire longitudinal-record ownership.

21.12.12.11.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.12.12.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

21.12.12.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-D42C14E5` (line 5132). Working locators are not identity aliases.

21.12.13. Function: Establish Encounter Context

Property	Value	Meaning
Reference Id	`HCM-FN-establish-encounter-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-ESTABLISH-ENCOUNTER-CONTEXT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-ESTABLISH-ENCOUNTER-CONTEXT`	Direct to this identity; never reusable.
Evidence	sa:68 (p. 1442)	Source standing does not approve the derived assertion.

21.12.13.1. Definition / governed semantic detail

Binds patient, attending practitioner, clinical class (e.g., Inpatient, Emergency, Ambulatory), and admitting diagnosis to a unique encounter identifier.

21.12.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Encounter Context Creation (p. 373)	Established

21.12.14. Service: Encounter Context Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-encounter-context-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-CONTEXT-RESOLUTION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-CONTEXT-RESOLUTION`	Direct to this identity; never reusable.
Evidence	sa:69 (p. 1442)	Source standing does not approve the derived assertion.

21.12.14.1. Definition / governed semantic detail

Discloses active encounter details to clinical applications.

21.12.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Encounter Context Creation (p. 373)	Established

21.12.15. Function: Progress Encounter State

Property	Value	Meaning
Reference Id	`HCM-FN-progress-encounter-state`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-PROGRESS-ENCOUNTER-STATE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-PROGRESS-ENCOUNTER-STATE`	Direct to this identity; never reusable.
Evidence	sa:71 (p. 1442); sa:74 (p. 1442)	Source standing does not approve the derived assertion.

21.12.15.1. Definition / governed semantic detail

Coordinates encounter progression (**scoped illustration; ON_LEAVE relationship unresolved**): *Planned → Arrived → In-Progress → Discharged → Completed*.

21.12.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Encounter State Progression (p. 374)	Established
To: governed process	Process: Encounter Lifecycle Process (p. 381)	Established

21.12.16. Service: Encounter Lifecycle Event Notification

Property	Value	Meaning
Reference Id	`HCM-SV-encounter-lifecycle-event-notification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LIFECYCLE-EVENT-NOTIFICATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LIFECYCLE-EVENT-NOTIFICATION`	Direct to this identity; never reusable.
Evidence	sa:72 (p. 1442); sa:74 (p. 1442)	Source standing does not approve the derived assertion.

21.12.16.1. Definition / governed semantic detail

Emits encounter transition events across the HIE.

21.12.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Encounter State Progression (p. 374)	Established

21.12.17. Function: Maintain Encounter Care-Place Association

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-encounter-care-place-association`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-MAINTAIN-ENCOUNTER-CARE-PLACE-ASSOCIATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-MAINTAIN-ENCOUNTER-CARE-PLACE-ASSOCIATION`	Direct to this identity; never reusable.
Evidence	sa:76 (p. 1442)	Source standing does not approve the derived assertion.

21.12.17.1. Definition / governed semantic detail

Binds the encounter to physical wards, rooms, and beds over time.

21.12.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Encounter Bed/Location Association (p. 375)	Established

21.12.18. Service: Encounter Location History Query

Property	Value	Meaning
Reference Id	`HCM-SV-encounter-location-history-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LOCATION-HISTORY-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LOCATION-HISTORY-QUERY`	Direct to this identity; never reusable.
Evidence	sa:77 (p. 1442)	Source standing does not approve the derived assertion.

21.12.18.1. Definition / governed semantic detail

Discloses patient location tracking history.

21.12.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Encounter Bed/Location Association (p. 375)	Established

21.12.19. Process: Encounter Lifecycle Process

Property	Value	Meaning
Reference Id	`HCM-PR-encounter-lifecycle-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.PR-ENCOUNTER-LIFECYCLE-PROCESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Episode & Encounter Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Episode & Encounter Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.PR-ENCOUNTER-LIFECYCLE-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:119 (p. 1417); processes:120 (p. 1417)	Source standing does not approve the derived assertion.

21.12.19.1. Process Overview

The encounter lifecycle distinguishes the planned opportunity, presence, active encounter and locally applicable completion context. It supplies an administrative progression frame for care information rather than evidence that every clinical objective has been met. The relation to Strategy's ON_LEAVE state remains unresolved, and local signing or legal-closure descriptions do not become universal completion rules.

21.12.19.2. Definition / governed semantic detail

- **Owning Capability**: `Episode&EncounterAdministration`
- **Process Purpose**: Governs the clinical and administrative lifecycle of an acute, emergency, inpatient, or outpatient encounter between a patient and healthcare services.
- **State Progression Lifecycle**: Planned / Booked → Arrived → Triaged / Ingested → Active In-Progress → Discharged → Completed / Encoded (Scoped illustration; ON_LEAVE relationship unresolved)
- **Key State Dispositions**:
  - `Planned/Booked`: Elective admission or clinic appointment scheduled.
  - `Arrived`: Patient presents at facility or emergency desk.
  - `Triaged/Ingested`: Clinical triage category assigned; encounter record activated.
  - `ActiveIn-Progress`: Inpatient care, bedside monitoring, and clinical orders actively underway.
  - `Discharged`: Patient clinically discharged; care-place vacated.
  - `Completed/Encoded`: Clinical documentation finalized, ICD/DRG coding completed, and encounter legally closed.
- **Scope qualification**: These completion criteria retain their locally described applicability; they do not establish universal signing, legal closure or originating authority over clinical information. Finalisation, signing and legal qualification remain distinct. The relationship with Strategy `ON_LEAVE`, including applicability and transitions, remains unresolved.

21.12.19.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Progress Encounter State (p. 378)	Established

21.13. BEC CT2: Order Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-order-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:245 (p. 1369); enabling:301 (p. 1371); sa:83 (p. 1443)	Source standing does not approve the derived assertion.

21.13.1. Capability Overview

Order Administration manages the closed loop around a request for defined fulfilment. It receives the request, resolves and routes to a destination, distinguishes delivery from business disposition, tracks progress and associates the eventual outcome. This makes the difference between an order sent and an order fulfilled visible throughout the requester's interaction.

21.13.2. Definition — Reviewed candidate

Order Administration manages the closed loop around a request for defined fulfilment. It receives the request, resolves and routes to a destination, distinguishes delivery from business disposition, tracks progress and associates the eventual outcome. This makes the difference between an order sent and an order fulfilled visible throughout the requester's interaction.

21.13.3. Intent — retained v0.1 source explanation

Preserve an intelligible connection between the original request, its destination, progression and returned outcome.

21.13.4. Boundary — retained v0.1 source explanation

Technical acknowledgements have only their contractual meaning. Receipt does not universally establish durable acceptance, business acceptance or completion; outcome association has no invented Feature equivalence.

21.13.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Reliable routing and closed-loop state are direct platform responsibilities. The performing service retains authority over clinical execution and its result.

21.13.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.13.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.13.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.13.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Receive Order Request (p. 391)	function	Canonical / Established
Service: Order Requisition Ingress (p. 391)	service	Canonical / Established
Function: Resolve Order Destination (p. 392)	function	Canonical / Established
Service: Order Dispatch Service (p. 393)	service	Canonical / Established
Function: Manage Order Progression (p. 393)	function	Canonical / Established
Service: Order Status & Tracking Query (p. 394)	service	Canonical / Established
Function: Coordinate Order Modification / Cancellation (p. 395)	function	Canonical / Established
Service: Order Cancellation Service (p. 396)	service	Canonical / Established
Function: Associate Order Outcome (p. 396)	function	Canonical / Established
Service: Order Outcome Notification (p. 397)	service	Canonical / Established
Process: Closed-Loop Order Progression Process (p. 398)	process	Canonical / Established
BEC CT3: Closed-Loop Order Progression (p. 385)	capability CT3	Proposed / Proposed

21.13.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Diagnostic Support Provision (p. 51)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed

21.13.11. BEC CT3: Closed-Loop Order Progression

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-closed-loop-order-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.13.11.1. Capability Overview

Administer the traceable progression of an order from request through destination, delivery, acknowledgement, fulfilment and outcome association. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.13.11.2. Definition — Reviewed candidate

Administer the traceable progression of an order from request through destination, delivery, acknowledgement, fulfilment and outcome association.

21.13.11.3. Intent

Administer the traceable progression of an order from request through destination, delivery, acknowledgement, fulfilment and outcome association.

21.13.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.13.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.13.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-5E9ECEA1` (line 5155). Working locators are not identity aliases.

21.13.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Order Request Ingestion (p. 386)	feature FT	Canonical / Proposed
Feature: Order Destination Resolution & Routing (p. 387)	feature FT	Canonical / Proposed
Feature: Order Closed-Loop Progression Tracking (p. 388)	feature FT	Canonical / Proposed
Feature: Order Cancellation & Modification Coordination (p. 389)	feature FT	Canonical / Proposed
Feature: Order Outcome Association (p. 390)	feature FT	Proposed / Proposed

21.13.11.8. Feature: Order Request Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-11`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-11`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration → BEC CT3: Closed-Loop Order Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-11`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Order Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:303 (p. 1371)	Source standing does not approve the derived assertion.

21.13.11.8.1. Feature Overview

Order ingestion establishes the received bounded fulfilment request with its ordering-clinician context. It begins the information account needed for a closed loop while leaving the per-

forming service's business acceptance and execution distinguishable from technical ingress acceptance.

21.13.11.8.2. Definition / governed semantic detail

Ingest clinical diagnostic, medication, and procedure order requests with validated ordering clinician context.

21.13.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.13.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Order Request (p. 391)	Established
From: associated feature	Service: Order Requisition Ingress (p. 391)	Established

21.13.11.9. Feature: Order Destination Resolution & Routing

Property	Value	Meaning
Reference Id	FEAT-SA-12	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-12	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration → BEC CT3: Closed-Loop Order Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-12	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Order Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:304 (p. 1371)	Source standing does not approve the derived assertion.

21.13.11.9.1. Feature Overview

Destination resolution connects an order with the service endpoint intended to fulfil it. Routing advances the request towards that destination without proving execution or changing the original clinical intent simply because a transport route was chosen.

21.13.11.9.2. Definition / governed semantic detail

Resolve fulfillment endpoints and reliably route orders to performing service systems.

21.13.11.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.13.11.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Resolve Order Destination (p. 392)	Established
From: associated feature	Service: Order Dispatch Service (p. 393)	Established

21.13.11.10. Feature: Order Closed-Loop Progression Tracking

Property	Value	Meaning
Reference Id	`FEAT-SA-13`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-13`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration → BEC CT3: Closed-Loop Order Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-13`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Order Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:305 (p. 1371)	Source standing does not approve the derived assertion.

21.13.11.10.1. Feature Overview

Closed-loop tracking keeps delivery, technical acknowledgement, business disposition, execution and returned outcome distinct. Its purpose is to expose what has actually been established at each stage, preventing a successful handoff from being reported as eventual fulfilment.

21.13.11.10.2. Definition / governed semantic detail

Track order progression across destination determination, routing, delivery, receipt acknowledgement (distinguishing technical delivery acknowledgement from business acknowledgement), execution progress, completion, and outcome association.

21.13.11.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.13.11.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Order Progression (p. 393)	Established
From: associated feature	Service: Order Status & Tracking Query (p. 394)	Established

21.13.11.11. Feature: Order Cancellation & Modification Coordination

Property	Value	Meaning
Reference Id	`FEAT-SA-14`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-14`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration → BEC CT3: Closed-Loop Order Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-14`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Order Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:306 (p. 1371)	Source standing does not approve the derived assertion.

21.13.11.11.1. Feature Overview

Cancellation and amendment coordination carries a requested change into the performing-system context. It preserves the difference between asking to change an order and evidence that the change took effect, rather than silently rewriting the original request as though no prior intent existed.

21.13.11.11.2. Definition / governed semantic detail

Coordinate order cancellation and amendment requests, ensuring synchronization with performing systems.

21.13.11.11.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.13.11.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Coordinate Order Modification / Cancellation (p. 395)	Established
From: associated feature	Service: Order Cancellation Service (p. 396)	Established

21.13.11.12. Feature: Order Outcome Association

Property	Value	Meaning
Reference Id	`HCM-FT-order-outcome-association`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-STEP2-ORDER-OUTCOME-ASSOCIATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration → BEC CT3: Closed-Loop Order Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-STEP2-ORDER-OUTCOME-ASSOCIATION`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

21.13.11.12.1. Feature Overview

Execution completion and returned outcome are different checkpoints. This Feature makes the result link explicit without equating an order acknowledgement with an outcome.

21.13.11.12.2. Definition — Proposed

Associate an externally established outcome with the applicable bounded order and its progression.

21.13.11.12.3. Boundary

Does not interpret diagnostic findings or declare fulfilment from transport delivery alone.

21.13.11.12.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.13.11.12.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

21.13.11.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Associate Order Outcome (p. 396)	Proposed

21.13.12. Function: Receive Order Request

Property	Value	Meaning
Reference Id	`HCM-FN-receive-order-request`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-RECEIVE-ORDER-REQUEST`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-RECEIVE-ORDER-REQUEST`	Direct to this identity; never reusable.
Evidence	sa:89 (p. 1443)	Source standing does not approve the derived assertion.

21.13.12.1. Definition / governed semantic detail

Ingests structured orders containing clinical indication, requesting clinician, and test specifications.

21.13.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Order Request Ingestion (p. 386)	Established

21.13.13. Service: Order Requisition Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-order-requisition-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-REQUISITION-INGRESS`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-REQUISITION-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:90 (p. 1443)	Source standing does not approve the derived assertion.

21.13.13.1. Definition / governed semantic detail

Ingests electronic orders from clinical ordering systems.

21.13.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Order Request Ingestion (p. 386)	Established

21.13.14. Function: Resolve Order Destination

Property	Value	Meaning
Reference Id	`HCM-FN-resolve-order-destination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-RESOLVE-ORDER-DESTINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-RESOLVE-ORDER-DESTINATION`	Direct to this identity; never reusable.
Evidence	sa:92 (p. 1443)	Source standing does not approve the derived assertion.

21.13.14.1. Definition / governed semantic detail

Evaluates order routing rules to determine target laboratory, imaging centre, or procedural unit.

21.13.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Order Destination Resolution & Routing (p. 387)	Established

21.13.15. Service: Order Dispatch Service

Property	Value	Meaning
Reference Id	`HCM-SV-order-dispatch-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-DISPATCH-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-DISPATCH-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:93 (p. 1443)	Source standing does not approve the derived assertion.

21.13.15.1. Definition / governed semantic detail

Transmits orders to performing diagnostic systems.

21.13.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Order Destination Resolution & Routing (p. 387)	Established

21.13.16. Function: Manage Order Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-order-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-MANAGE-ORDER-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-MANAGE-ORDER-PROGRESSION`	Direct to this identity; never reusable.
Evidence	sa:95 (p. 1443); sa:98 (p. 1443)	Source standing does not approve the derived assertion.

21.13.16.1. Definition / governed semantic detail

Tracks order state (**scoped diagnostic/procedural illustration**): *Placed → Received → Specimen Collected → In-Progress → Preliminary Result → Final Result → Closed*.

21.13.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Order Closed-Loop Progression Tracking (p. 388)	Established
To: governed process	Process: Closed-Loop Order Progression Process (p. 398)	Established

21.13.17. Service: Order Status & Tracking Query

Property	Value	Meaning
Reference Id	`HCM-SV-order-status-tracking-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-STATUS-TRACKING-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-STATUS-TRACKING-QUERY`	Direct to this identity; never reusable.
Evidence	sa:96 (p. 1443); sa:98 (p. 1443)	Source standing does not approve the derived assertion.

21.13.17.1. Definition / governed semantic detail

Provides real-time order tracking to ordering clinicians.

21.13.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Order Closed-Loop Progression Tracking (p. 388)	Established

21.13.18. Function: Coordinate Order Modification / Cancellation

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-order-modification-cancellation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-COORDINATE-ORDER-MODIFICATION-CANCELLATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-COORDINATE-ORDER-MODIFICATION-CANCELLATION`	Direct to this identity; never reusable.
Evidence	sa:100 (p. 1443)	Source standing does not approve the derived assertion.

21.13.18.1. Definition / governed semantic detail

Handles requests to amend clinical details or cancel unexecuted orders.

21.13.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Order Cancellation & Modification Coordination (p. 389)	Established

21.13.19. Service: Order Cancellation Service

Property	Value	Meaning
Reference Id	`HCM-SV-order-cancellation-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-CANCELLATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-CANCELLATION-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:101 (p. 1443)	Source standing does not approve the derived assertion.

21.13.19.1. Definition / governed semantic detail

Dispatches cancel directives to performing systems.

21.13.19.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Order Cancellation & Modification Coordination (p. 389)	Established

21.13.20. Function: Associate Order Outcome

Property	Value	Meaning
Reference Id	`HCM-FN-associate-order-outcome`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-ASSOCIATE-ORDER-OUTCOME`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-ASSOCIATE-ORDER-OUTCOME`	Direct to this identity; never reusable.
Evidence	sa:103 (p. 1443); sa:102 (p. 1443)	Source standing does not approve the derived assertion.

21.13.20.1. Definition / governed semantic detail

Correlates returning diagnostic reports and observations back to the originating order requisition.

21.13.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Order Outcome Association (p. 390)	Proposed

21.13.21. Service: Order Outcome Notification

Property	Value	Meaning
Reference Id	`HCM-SV-order-outcome-notification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-OUTCOME-NOTIFICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-OUTCOME-NOTIFICATION`	Direct to this identity; never reusable.
Evidence	sa:104 (p. 1443); sa:102 (p. 1443)	Source standing does not approve the derived assertion.

21.13.21.1. Definition / governed semantic detail

Emits completion notices to ordering practitioners.

21.13.22. Process: Closed-Loop Order Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-closed-loop-order-progression-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.PR-CLOSED-LOOP-ORDER-PROGRESSION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Order Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.PR-CLOSED-LOOP-ORDER-PROGRESSION-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:135 (p. 1417); processes:136 (p. 1417)	Source standing does not approve the derived assertion.

21.13.22.1. Process Overview

Closed-loop progression preserves the connection between the requisition, performing context and returned result so that a request does not disappear after dispatch. The diagnostic/procedural illustration includes specimen and report checkpoints without excluding medication from the broader owning capability. Dispatch, recipient acceptance, result binding and requester closure remain distinct facts even where a source checkpoint combines them.

21.13.22.2. Definition / governed semantic detail

- **Owning Capability**: `OrderAdministration`
- **Process Purpose**: Governs the rigorous, closed-loop tracking of diagnostic pathology, radiology, and procedural orders from requisition to result correlation, preventing dropped or unfulfilled investigations.
- **Scope qualification**: These examples describe a diagnostic/procedural progression and do not exclude medication from the broader Order Administration responsibility. Specimen and result checkpoints are scoped. The signed requisition/report requirements remain within this illustrated scope; signing is not universally equivalent to finalisation or authority. Result consumption/binding does not define originating validity, authority, authorship or legal status.
- **State Progression Lifecycle**: Requisition Placed → Order Dispatched → Specimen Collected / Scheduled → In-Execution → Preliminary Result Bound → Final Result Bound → Closed / Verified (Scoped diagnostic/procedural illustration)
- **Key State Dispositions**:
  - `RequisitionPlaced`: Electronic order created and signed by requesting clinician.

- `OrderDispatched`: Order routed and accepted by performing diagnostic service. This local compound checkpoint retains routing and recipient acceptance as distinct facts; dispatch alone establishes neither receipt nor acceptance. Behaviour `Received` is not equated with this checkpoint without evidence.
- `SpecimenCollected/Scheduled`: Bio-specimen collected or imaging appointment booked.
- `In-Execution`: Laboratory analysis or diagnostic scanning underway.
- `PreliminaryResultBound`: Critical or interim observations published and linked to order.
- `FinalResultBound`: Authoritative diagnostic report signed and bound to order.
- `Closed/Verified`: Requesting clinician acknowledges result; order closed. This Order-closure checkpoint does not establish diagnostic-content verification, approval or clinical incorporation.

21.13.22.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Order Progression (p. 393)	Established

21.14. BEC CT2: Diagnostic Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-diagnostic-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:246 (p. 1369); enabling:308 (p. 1371); sa:110 (p. 1444)	Source standing does not approve the derived assertion.

21.14.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.14.2. Capability Overview

Diagnostic Administration correlates requests with specimens and encounters and integrates diagnostic reports into their request context. It connects the laboratory or imaging contribution with the participants expecting an outcome, preserving report identity and standing as updates occur. Its enabling role differs from performing the diagnostic procedure or professionally interpreting evidence.

21.14.3. Definition — Reviewed candidate

Diagnostic Administration correlates requests with specimens and encounters and integrates diagnostic reports into their request context. It connects the laboratory or imaging contribution with the participants expecting an outcome, preserving report identity and standing as updates occur. Its enabling role differs from performing the diagnostic procedure or professionally interpreting evidence.

21.14.4. Boundary — retained v0.1 source explanation

The finalised-report distribution Feature retains its narrower scope. Broader Domain03 behaviour covering preliminary, corrected or addendum information does not silently expand that Feature.

21.14.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Request, specimen and report correlation and governed distribution are substantive information-integration responsibilities.

21.14.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.14.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.14.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.14.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Diagnostic Service Integration (p. 401)	capability CT3	Canonical / Proposed
Function: Correlate Diagnostic Request (p. 413)	function	Canonical / Established
Service: Diagnostic Correlation Resolution (p. 414)	service	Canonical / Established
Function: Bind Diagnostic Report (p. 414)	function	Canonical / Established
Service: Diagnostic Report Ingress (p. 415)	service	Canonical / Established

Element / page	Type / tier	Element / placement status
Function: Distribute Diagnostic Report (p. 416)	function	Canonical / Established
Service: Diagnostic Result Delivery Service (p. 417)	service	Canonical / Established
BEC CT3: Diagnostic Workflow Management (p. 408)	capability CT3	Proposed / Proposed

21.14.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Diagnostic Support Provision (p. 51)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed

21.14.11. BEC CT3: Diagnostic Service Integration

Property	Value	Meaning
Reference Id	`HCM-BEC-diagnostic-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES`	Direct to this identity; never reusable.
Established name	Diagnostic Services	Current display rename is Proposed; meaning unchanged.
Name history	Diagnostic Services	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:368 (p. 1372); enabling:409 (p. 1374); sd:129 (p. 1451)	Source standing does not approve the derived assertion.

21.14.11.1. Capability Overview

Diagnostic Services supports cross-provider availability of reports and diagnostic history. Result distribution brings established findings to the ordering and designated participants; history consolidation makes relevant evidence visible across institutional boundaries. This delivery context complements Diagnostic Administration's request and report associations without making information integration the professional diagnostic service itself.

21.14.11.2. Boundary

Consolidating trends does not interpret diagnostic evidence or establish a patient-level clinical conclusion.

21.14.11.3. Harmonia Architectural Relevance

Governed report distribution and integrated history materially enable diagnostic information use across care boundaries.

21.14.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.14.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.14.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.14.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Laboratory & Imaging Result Distribution (p. 403)	feature FT	Canonical / Proposed
Feature: Diagnostic History Consolidation (p. 404)	feature FT	Canonical / Proposed
Function: Receive Diagnostic Requisition Feed (p. 404)	function	Canonical / Established
Service: Modality Worklist Distribution (p. 405)	service	Canonical / Established
Function: Publish Governed Diagnostic Report (p. 406)	function	Canonical / Established
Service: Diagnostic Publication Service (p. 407)	service	Canonical / Established

21.14.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Diagnostic Support Provision (p. 51)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

21.14.11.9. Feature: Laboratory & Imaging Result Distribution

Property	Value	Meaning
Reference Id	`FEAT-SD-09`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-09`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Service Integration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-09`	Direct to this identity; never reusable.
Evidence	enabling:411 (p. 1374)	Source standing does not approve the derived assertion.

21.14.11.9.1. Feature Overview

Result distribution connects established diagnostic reports with ordering and designated secondary recipients. It supports continuity of diagnostic information across care settings while retaining the distinction between delivery, recipient review and the clinical decision based on the result.

21.14.11.9.2. Definition / governed semantic detail

Reliably distribute structured diagnostic reports (pathology, radiology) to ordering clinicians and designated secondary recipients.

21.14.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Diagnostic Requisition Feed (p. 404)	Unresolved association
From: associated feature	Service: Modality Worklist Distribution (p. 405)	Unresolved association

21.14.11.10. Feature: Diagnostic History Consolidation

Property	Value	Meaning
Reference Id	`FEAT-SD-10`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-10`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Service Integration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-10`	Direct to this identity; never reusable.
Evidence	enabling:412 (p. 1374)	Source standing does not approve the derived assertion.

21.14.11.10.1. Feature Overview

Diagnostic history consolidation makes prior evidence visible across regional providers. A combined historical view can support comparison over time without itself establishing a diagnostic interpretation, reconciling conflicting clinical assertions or replacing each report's source standing.

21.14.11.10.2. Definition / governed semantic detail

Consolidate historical diagnostic trends (e.g., cumulative laboratory results) across multiple regional providers into an integrated view.

21.14.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Publish Governed Diagnostic Report (p. 406)	Unresolved association
From: associated feature	Service: Diagnostic Publication Service (p. 407)	Unresolved association

21.14.11.11. Function: Receive Diagnostic Requisition Feed

Property	Value	Meaning
Reference Id	`HCM-FN-receive-diagnostic-requisition-feed`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-RECEIVE-DIAGNOSTIC-REQUISITION-FEED`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Service Integration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-RECEIVE-DIAGNOSTIC-REQUISITION-FEED`	Direct to this identity; never reusable.
Evidence	sd:134 (p. 1451); sd:133 (p. 1451)	Source standing does not approve the derived assertion.

21.14.11.11.1. Definition / governed semantic detail

Normalises order requisitions from across the health network into modality worklists.

Feature association not established: Strategy `FEAT-SD-09 — Laboratory & Imaging Result Distribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.14.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Laboratory & Imaging Result Distribution (p. 403)	Unresolved association

21.14.11.12. Service: Modality Worklist Distribution

Property	Value	Meaning
Reference Id	`HCM-SV-modality-worklist-distribution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-MODALITY-WORKLIST-DISTRIBUTION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Service Integration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-MODALITY-WORKLIST-DISTRIBUTION`	Direct to this identity; never reusable.
Evidence	sd:135 (p. 1451); sd:133 (p. 1451)	Source standing does not approve the derived assertion.

21.14.11.12.1. Definition / governed semantic detail

Supplies order worklists to diagnostic laboratory/imaging systems.

Feature association not established: Strategy `FEAT-SD-09 — Laboratory & Imaging Result Distribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.14.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Laboratory & Imaging Result Distribution (p. 403)	Unresolved association

21.14.11.13. Function: Publish Governed Diagnostic Report

Property	Value	Meaning
Reference Id	`HCM-FN-publish-governed-diagnostic-report`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-PUBLISH-GOVERNED-DIAGNOSTIC-REPORT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Service Integration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-PUBLISH-GOVERNED-DIAGNOSTIC-REPORT`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	sd:138 (p. 1451); sd:137 (p. 1451)	Source standing does not approve the derived assertion.

21.14.11.13.1. Definition / governed semantic detail

Normalises and distributes structured diagnostic observations, reference ranges, and diagnostic image study links into the LHR.

Feature association not established: Strategy `FEAT-SD-10 — Diagnostic History Consolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.14.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Diagnostic History Consolidation (p. 404)	Unresolved association

21.14.11.14. Service: Diagnostic Publication Service

Property	Value	Meaning
Reference Id	`HCM-SV-diagnostic-publication-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-DIAGNOSTIC-PUBLICATION-SERVICE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Service Integration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-DIAGNOSTIC-PUBLICATION-SERVICE`	Direct to this identity; never reusable.
Evidence	sd:139 (p. 1451); sd:137 (p. 1451)	Source standing does not approve the derived assertion.

21.14.11.14.1. Definition / governed semantic detail

Publishes verified diagnostic reports across the enterprise.

Feature association not established: Strategy `FEAT-SD-10 — Diagnostic History Consolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.14.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Diagnostic History Consolidation (p. 404)	Unresolved association

21.14.12. BEC CT3: Diagnostic Workflow Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-diagnostic-workflow-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.14.12.1. Capability Overview

Administer the established diagnostic workflow management information and coordination responsibility while preserving its source authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.14.12.2. Definition — Proposed

Administer the established diagnostic workflow management information and coordination responsibility while preserving its source authority.

21.14.12.3. Intent

Administer the established diagnostic workflow management information and coordination responsibility while preserving its source authority.

21.14.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.14.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.14.12.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-C0DD4810` (line 5278). Working locators are not identity aliases.

21.14.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Diagnostic Request Correlation (p. 409)	feature FT	Canonical / Proposed
Feature: Diagnostic Report Ingestion & Binding (p. 410)	feature FT	Canonical / Proposed
Feature: Diagnostic Report Distribution (p. 411)	feature FT	Canonical / Proposed
Feature: Diagnostic Preliminary & Corrected Report Handling (p. 412)	feature FT	Proposed / Proposed

21.14.12.8. Feature: Diagnostic Request Correlation

Property	Value	Meaning
Reference Id	`FEAT-SA-15`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-15`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Workflow Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-SA-15`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Diagnostic Administration	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:310 (p. 1371)	Source standing does not approve the derived assertion.

21.14.12.8.1. Feature Overview

Diagnostic request correlation connects the order with its specimen and encounter context. It helps distinguish evidence belonging to the intended fulfilment from superficially similar results, without treating association as proof of specimen suitability or diagnostic interpretation.

21.14.12.8.2. Definition / governed semantic detail

Correlate diagnostic test orders with collected clinical specimens and patient encounters.

21.14.12.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.14.12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Correlate Diagnostic Request (p. 413)	Established
From: associated feature	Service: Diagnostic Correlation Resolution (p. 414)	Established

21.14.12.9. Feature: Diagnostic Report Ingestion & Binding

Property	Value	Meaning
Reference Id	FEAT-SA-16	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-16	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Workflow Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-SA-16	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Diagnostic Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:311 (p. 1371)	Source standing does not approve the derived assertion.

21.14.12.9.1. Feature Overview

Report ingestion binds a finalised diagnostic contribution to the order it answers and verifies content integrity. The report's professional standing comes from its diagnostic source; successful ingestion does not independently authorise its findings or establish a patient-level clinical conclusion.

21.14.12.9.2. Definition / governed semantic detail

Ingest finalized laboratory and radiology reports, verifying content integrity and binding them to the parent order.

21.14.12.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.14.12.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Bind Diagnostic Report (p. 414)	Established
From: associated feature	Service: Diagnostic Report Ingress (p. 415)	Established

21.14.12.10. Feature: Diagnostic Report Distribution

Property	Value	Meaning
Reference Id	`FEAT-SA-17`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-17`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Workflow Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-SA-17`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Diagnostic Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:312 (p. 1371)	Source standing does not approve the derived assertion.

21.14.12.10.1. Feature Overview

This Feature distributes finalised diagnostic reports to the ordering and designated care participants. Its narrower finalised-report scope is preserved even though broader Diagnostic Ad-

ministration behaviour also handles preliminary, corrected and addendum information; those behaviours do not silently redefine the Feature.

21.14.12.10.2. Definition / governed semantic detail

Distribute finalized laboratory, pathology, and imaging reports to ordering clinicians and designated care teams.

21.14.12.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.14.12.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Distribute Diagnostic Report (p. 416)	Established
From: associated feature	Service: Diagnostic Result Delivery Service (p. 417)	Established

21.14.12.11. Feature: Diagnostic Preliminary & Corrected Report Handling

Property	Value	Meaning
Reference Id	`HCM-FT-diagnostic-preliminary-corrected-report-handling`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-STEP2-DIAGNOSTIC-PRELIMINARY-CORRECTED-REPORT-HANDLING`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration → BEC CT3: Diagnostic Workflow Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-STEP2-DIAGNOSTIC-PRELIMINARY-CORRECTED-REPORT-HANDLING`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10; 02-service-administration.md:109	Source standing does not approve the derived assertion.

21.14.12.11.1. Feature Overview

The finalised-report Feature remains unchanged. The broader documented diagnostic behaviour needs a separate frame for information whose preliminary or correction standing is material.

21.14.12.11.2. Definition — Proposed

Receive, qualify and distribute preliminary, corrected and addendum diagnostic information with its relationship to prior reports.

21.14.12.11.3. Boundary

Does not originate diagnostic interpretation or silently overwrite a previously received report.

21.14.12.11.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.14.12.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10; 02-service-administration.md:109

21.14.13. Function: Correlate Diagnostic Request

Property	Value	Meaning
Reference Id	`HCM-FN-correlate-diagnostic-request`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-CORRELATE-DIAGNOSTIC-REQUEST`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FN-CORRELATE-DIAGNOSTIC-REQUEST`	Direct to this identity; never reusable.
Evidence	sa:116 (p. 1444)	Source standing does not approve the derived assertion.

21.14.13.1. Definition / governed semantic detail

Matches incoming lab/radiology results to active patient encounters and order requisitions.

21.14.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Diagnostic Request Correlation (p. 409)	Established

21.14.14. Service: Diagnostic Correlation Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-diagnostic-correlation-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-CORRELATION-RESOLUTION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-CORRELATION-RESOLUTION`	Direct to this identity; never reusable.
Evidence	sa:117 (p. 1444)	Source standing does not approve the derived assertion.

21.14.14.1. Definition / governed semantic detail

Discloses correlated order/result linkages.

21.14.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Diagnostic Request Correlation (p. 409)	Established

21.14.15. Function: Bind Diagnostic Report

Property	Value	Meaning
Reference Id	`HCM-FN-bind-diagnostic-report`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-BIND-DIAGNOSTIC-REPORT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FN-BIND-DIAGNOSTIC-REPORT`	Direct to this identity; never reusable.
Evidence	sa:119 (p. 1444)	Source standing does not approve the derived assertion.

21.14.15.1. Definition / governed semantic detail

Associates structured observations, reference ranges, and abnormal flags with patient health records.

21.14.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Diagnostic Report Ingestion & Binding (p. 410)	Established

21.14.16. Service: Diagnostic Report Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-diagnostic-report-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-REPORT-INGRESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-REPORT-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:120 (p. 1444)	Source standing does not approve the derived assertion.

21.14.16.1. Definition / governed semantic detail

Ingests results from external LIS/RIS systems.

21.14.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Diagnostic Report Ingestion & Binding (p. 410)	Established

21.14.17. Function: Distribute Diagnostic Report

Property	Value	Meaning
Reference Id	`HCM-FN-distribute-diagnostic-report`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-DISTRIBUTE-DIAGNOSTIC-REPORT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FN-DISTRIBUTE-DIAGNOSTIC-REPORT`	Direct to this identity; never reusable.
Evidence	sa:122 (p. 1444); sa:123 (p. 1444)	Source standing does not approve the derived assertion.

21.14.17.1. Definition / governed semantic detail

Delivers preliminary, final, and corrected reports to ordering and copied practitioners.

21.14.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Diagnostic Report Distribution (p. 411)	Established

21.14.18. Service: Diagnostic Result Delivery Service

Property	Value	Meaning
Reference Id	`HCM-SV-diagnostic-result-delivery-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-RESULT-DELIVERY-SERVICE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Diagnostic Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Diagnostic Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-RESULT-DELIVERY-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:124 (p. 1444); sa:123 (p. 1444)	Source standing does not approve the derived assertion.

21.14.18.1. Definition / governed semantic detail

Emits secure result notifications.

21.14.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Diagnostic Report Distribution (p. 411)	Established

21.15. BEC CT2: Medication Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-medication-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:247 (p. 1369); enabling:316 (p. 1371); sa:130 (p. 1444)	Source standing does not approve the derived assertion.

21.15.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.15.2. Capability Overview

In this enabling catalogue, Medication Administration integrates medication orders, dispensing evidence and point-of-care administration records. It preserves the distinct events through which intended therapy becomes supplied and, where reported, administered. This administrative integration supports a coherent medication account across systems without itself dispensing or administering a medicine.

21.15.3. Definition — Reviewed candidate

In this enabling catalogue, Medication Administration integrates medication orders, dispensing evidence and point-of-care administration records. It preserves the distinct events through which intended therapy becomes supplied and, where reported, administered. This administrative integration supports a coherent medication account across systems without itself dispensing or administering a medicine.

21.15.4. Boundary — retained v0.1 source explanation

Prescribed, dispensed and administered are different states. The capability name does not assign bedside medication delivery to Harmonia.

21.15.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Connecting medication events materially supports continuity and interpretation while clinical and pharmacy actors retain originating responsibility.

21.15.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.15.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.15.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.15.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	capability CT3	Canonical / Proposed
Function: Receive Medication Order (p. 431)	function	Canonical / Established
Service: Medication Order Ingress (p. 431)	service	Canonical / Established
Function: Track Dispense Event (p. 432)	function	Canonical / Established
Service: Dispense Event Ingress (p. 433)	service	Canonical / Established
Function: Receive Medication Administration Event (p. 434)	function	Canonical / Established
Service: Medication Administration Ingress (p. 434)	service	Canonical / Established
BEC CT3: Medication Order & Dispense Management (p. 426)	capability CT3	Proposed / Proposed

21.15.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Medication & Pharmacy Services (p. 58)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed

21.15.11. BEC CT3: Medication Therapy Provision and Monitoring

Property	Value	Meaning
Reference Id	`HCM-BEC-medication-therapy`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY`	Direct to this identity; never reusable.
Established name	Medication Therapy	Current display rename is Proposed; meaning unchanged.
Name history	Medication Therapy	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:370 (p. 1372); enabling:414 (p. 1374); sd:147 (p. 1452)	Source standing does not approve the derived assertion.

21.15.11.1. Capability Overview

Medication Therapy connects reconciled histories and medication events across care transitions. The clinically established medication account is received and made available with order, dispensing and administration information still distinguishable. This supports continuity of therapy understanding without turning the platform into the clinician performing medication reconciliation or choosing treatment.

21.15.11.2. Boundary

Ingesting a reconciled history is not clinical reconciliation, and an order or dispense event does not prove administration.

21.15.11.3. Harmonia Architectural Relevance

Cross-boundary medication information materially supports care continuity and interpretation.

21.15.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.15.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.15.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.15.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Medication History Reconciliation Ingestion (p. 421)	feature FT	Canonical / Proposed
Feature: Medication Administration Tracking (p. 422)	feature FT	Canonical / Proposed
Function: Project Longitudinal Medication History (p. 423)	function	Canonical / Established
Service: Longitudinal Medication History Query (p. 424)	service	Canonical / Established
Function: Correlate Adverse Reaction & Allergy Profile (p. 424)	function	Canonical / Established
Service: Allergy & Adverse Reaction Query (p. 425)	service	Canonical / Established

21.15.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Medication & Pharmacy Services (p. 58)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

21.15.11.9. Feature: Medication History Reconciliation Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SD-11`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-11`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Therapy Provision and Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-11`	Direct to this identity; never reusable.
Evidence	enabling:416 (p. 1374)	Source standing does not approve the derived assertion.

21.15.11.9.1. Feature Overview

This Feature receives medication histories already reconciled by clinical participants during care transitions. The platform preserves and connects that account; ingestion is distinct from performing the clinical reconciliation or independently deciding which regimen is correct.

21.15.11.9.2. Definition / governed semantic detail

Ingest reconciled medication histories compiled during clinical transitions of care.

21.15.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Project Longitudinal Medication History (p. 423)	Established
From: associated feature	Service: Longitudinal Medication History Query (p. 424)	Established

21.15.11.10. Feature: Medication Administration Tracking

Property	Value	Meaning
Reference Id	`FEAT-SD-12`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-12`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Therapy Provision and Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-12`	Direct to this identity; never reusable.
Evidence	enabling:417 (p. 1374)	Source standing does not approve the derived assertion.

21.15.11.10.1. Feature Overview

Medication tracking conveys orders, dispensing changes and administration information across care boundaries while keeping their meanings separate. This supports a coherent therapy account without treating a supplied medicine as an administered dose or a distributed order as treatment performed.

21.15.11.10.2. Definition / governed semantic detail

Distribute medication therapy orders, dispensing updates, and administration records across clinical care boundaries.

21.15.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Correlate Adverse Reaction & Allergy Profile (p. 424)	Unresolved association
From: associated feature	Service: Allergy & Adverse Reaction Query (p. 425)	Unresolved association

21.15.11.11. Function: Project Longitudinal Medication History

Property	Value	Meaning
Reference Id	`HCM-FN-project-longitudinal-medication-history`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FN-PROJECT-LONGITUDINAL-MEDICATION-HISTORY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Therapy Provision and Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FN-PROJECT-LONGITUDINAL-MEDICATION-HISTORY`	Direct to this identity; never reusable.
Evidence	sd:151 (p. 1452)	Source standing does not approve the derived assertion.

21.15.11.11.1. Definition / governed semantic detail

Aggregates community dispensing, hospital e-MAR administrations, and specialist prescriptions into a unified chronological medication profile.

21.15.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Medication History Reconciliation Ingestion (p. 421)	Established

21.15.11.12. Service: Longitudinal Medication History Query

Property	Value	Meaning
Reference Id	`HCM-SV-longitudinal-medication-history-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.SV-LONGITUDINAL-MEDICATION-HISTORY-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Therapy Provision and Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.SV-LONGITUDINAL-MEDICATION-HISTORY-QUERY`	Direct to this identity; never reusable.
Evidence	sd:152 (p. 1452)	Source standing does not approve the derived assertion.

21.15.11.12.1. Definition / governed semantic detail

Discloses full medication timelines for medication reconciliation.

21.15.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Medication History Reconciliation Ingestion (p. 421)	Established

21.15.11.13. Function: Correlate Adverse Reaction & Allergy Profile

Property	Value	Meaning
Reference Id	`HCM-FN-correlate-adverse-reaction-allergy-profile`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FN-CORRELATE-ADVERSE-REACTION-ALLERGY-PROFILE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Therapy Provision and Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FN-CORRELATE-ADVERSE-REACTION-ALLERGY-PROFILE`	Direct to this identity; never reusable.
Evidence	sd:155 (p. 1452); sd:154 (p. 1452)	Source standing does not approve the derived assertion.

21.15.11.13.1. Definition / governed semantic detail

Binds confirmed substance allergies, adverse drug reactions, and severity grades to the patient identity.

Feature association not established: Strategy `FEAT-SD-12 — Medication Administration Tracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.15.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Medication Administration Tracking (p. 422)	Unresolved association

21.15.11.14. Service: Allergy & Adverse Reaction Query

Property	Value	Meaning
Reference Id	`HCM-SV-allergy-adverse-reaction-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.SV-ALLERGY-ADVERSE-REACTION-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Therapy Provision and Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.SV-ALLERGY-ADVERSE-REACTION-QUERY`	Direct to this identity; never reusable.
Evidence	sd:156 (p. 1452); sd:154 (p. 1452)	Source standing does not approve the derived assertion.

21.15.11.14.1. Definition / governed semantic detail

Exposes active allergy profiles to clinical decision support systems.

Feature association not established: Strategy `FEAT-SD-12 — Medication Administration Tracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.15.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Medication Administration Tracking (p. 422)	Unresolved association

21.15.12. BEC CT3: Medication Order & Dispense Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-medication-order-dispense-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.15.12.1. Capability Overview

Administer the established medication order & dispense management information and coordination responsibility while preserving its source authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.15.12.2. Definition — Proposed

Administer the established medication order & dispense management information and coordination responsibility while preserving its source authority.

21.15.12.3. Intent

Administer the established medication order & dispense management information and coordination responsibility while preserving its source authority.

21.15.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.15.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.15.12.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-1EFAA849` (line 5375). Working locators are not identity aliases.

21.15.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Medication Order Ingestion (p. 428)	feature FT	Canonical / Proposed
Feature: Dispense Event Tracking (p. 429)	feature FT	Canonical / Proposed
Feature: Medication Administration Record (MAR) Ingestion (p. 430)	feature FT	Canonical / Proposed

21.15.12.8. Feature: Medication Order Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-18`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-18`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Order & Dispense Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FEAT-SA-18`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Medication Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:318 (p. 1371)	Source standing does not approve the derived assertion.

21.15.12.8.1. Feature Overview

Medication order ingestion preserves the intended therapy expressed by inpatient or discharge prescriptions. The received order is evidence of clinical intent, distinct from pharmacy dispensing and the patient's subsequent administration record.

21.15.12.8.2. Definition / governed semantic detail

Ingest inpatient and discharge prescription orders.

21.15.12.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.15.12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Medication Order (p. 431)	Established
From: associated feature	Service: Medication Order Ingress (p. 431)	Established

21.15.12.9. Feature: Dispense Event Tracking

Property	Value	Meaning
Reference Id	FEAT-SA-19	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-19	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Order & Dispense Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FEAT-SA-19	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Medication Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:319 (p. 1371)	Source standing does not approve the derived assertion.

21.15.12.9.1. Feature Overview

Dispense tracking receives the pharmacy's evidence that medication was supplied under the applicable context. It connects supply with the medication account without inferring that the patient received a dose or that Harmonia performed the professional dispensing activity.

21.15.12.9.2. Definition / governed semantic detail

Ingest medication dispense verification records from pharmacy management systems.

21.15.12.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.15.12.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Track Dispense Event (p. 432)	Established
From: associated feature	Service: Dispense Event Ingress (p. 433)	Established

21.15.12.10. Feature: Medication Administration Record (MAR) Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-20`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-20`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration → BEC CT3: Medication Order & Dispense Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FEAT-SA-20`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Medication Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:320 (p. 1371)	Source standing does not approve the derived assertion.

21.15.12.10.1. Feature Overview

MAR ingestion captures source-reported point-of-care administration events. It preserves the crucial distinction between intended or supplied medication and an administration recorded as performed, without originating that bedside event or its clinical authority.

21.15.12.10.2. Definition / governed semantic detail

Ingest point-of-care medication administration events.

21.15.12.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.15.12.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Medication Administration Event (p. 434)	Established
From: associated feature	Service: Medication Administration Ingress (p. 434)	Established

21.15.13. Function: Receive Medication Order

Property	Value	Meaning
Reference Id	`HCM-FN-receive-medication-order`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ORDER`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ORDER`	Direct to this identity; never reusable.
Evidence	sa:136 (p. 1444)	Source standing does not approve the derived assertion.

21.15.13.1. Definition / governed semantic detail

Ingests inpatient and outpatient drug prescriptions.

21.15.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Medication Order Ingestion (p. 428)	Established

21.15.14. Service: Medication Order Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-medication-order-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ORDER-INGRESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ORDER-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:137 (p. 1444)	Source standing does not approve the derived assertion.

21.15.14.1. Definition / governed semantic detail

Ingests prescription events.

21.15.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Medication Order Ingestion (p. 428)	Established

21.15.15. Function: Track Dispense Event

Property	Value	Meaning
Reference Id	`HCM-FN-track-dispense-event`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-TRACK-DISPENSE-EVENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FN-TRACK-DISPENSE-EVENT`	Direct to this identity; never reusable.
Evidence	sa:139 (p. 1444)	Source standing does not approve the derived assertion.

21.15.15.1. Definition / governed semantic detail

Ingests community and hospital pharmacy dispensing records.

21.15.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Dispense Event Tracking (p. 429)	Established

21.15.16. Service: Dispense Event Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-dispense-event-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-DISPENSE-EVENT-INGRESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.SV-DISPENSE-EVENT-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:140 (p. 1444)	Source standing does not approve the derived assertion.

21.15.16.1. Definition / governed semantic detail

Ingests pharmacy supply notifications.

21.15.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Dispense Event Tracking (p. 429)	Established

21.15.17. Function: Receive Medication Administration Event

Property	Value	Meaning
Reference Id	`HCM-FN-receive-medication-administration-event`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ADMINISTRATION-EVENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ADMINISTRATION-EVENT`	Direct to this identity; never reusable.
Evidence	sa:142 (p. 1444)	Source standing does not approve the derived assertion.

21.15.17.1. Definition / governed semantic detail

Records medication administration-event information, including nurse-administered doses, e-MAR documentation and infusion telemetry as non-exhaustive examples; scope is not limited to nursing, bedside or inpatient settings.

21.15.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Medication Administration Record (MAR) Ingestion (p. 430)	Established

21.15.18. Service: Medication Administration Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-medication-administration-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ADMINISTRATION-INGRESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Medication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Medication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ADMINISTRATION-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:143 (p. 1444)	Source standing does not approve the derived assertion.

21.15.18.1. Definition / governed semantic detail

Ingests administration records.

21.15.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Medication Administration Record (MAR) Ingestion (p. 430)	Established

21.16. BEC CT2: Procedure Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-procedure-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION`	Direct to this identity; never reusable.

Property	Value	Meaning
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:248 (p. 1369); enabling:322 (p. 1371); sa:149 (p. 1445)	Source standing does not approve the derived assertion.

21.16.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.16.2. Capability Overview

Procedure Administration integrates procedural bookings and the documentation of procedures performed. It keeps the scheduled case distinct from operative, anaesthetic and post-procedure information received later. That separation supports continuity across scheduling and clinical documentation without inferring that a booking establishes performance.

21.16.3. Definition — Reviewed candidate

Procedure Administration integrates procedural bookings and the documentation of procedures performed. It keeps the scheduled case distinct from operative, anaesthetic and post-procedure information received later. That separation supports continuity across scheduling and clinical documentation without inferring that a booking establishes performance.

21.16.4. Boundary — retained v0.1 source explanation

Administrative integration neither performs the procedure nor originates its professional findings.

21.16.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Correlated bookings and procedural documents support cross-system care context while the surgical or procedural service retains clinical authority.

21.16.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.16.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.16.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.16.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Receive Procedure Booking / Request (p. 440)	function	Canonical / Established
Service: Procedure Booking Ingress (p. 441)	service	Canonical / Established
Function: Receive Procedural Documentation (p. 442)	function	Canonical / Established
Service: Procedural Documentation Ingress (p. 443)	service	Canonical / Established
BEC CT3: Procedure & Surgical Administration (p. 437)	capability CT3	Proposed / Proposed

21.16.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed

21.16.11. BEC CT3: Procedure & Surgical Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-procedure-surgical-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.16.11.1. Capability Overview

Administer the established procedure & surgical administration information and coordination responsibility while preserving its source authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.16.11.2. Definition — Proposed

Administer the established procedure & surgical administration information and coordination responsibility while preserving its source authority.

21.16.11.3. Intent

Administer the established procedure & surgical administration information and coordination responsibility while preserving its source authority.

21.16.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.16.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.16.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-489D3945` (line 5537). Working locators are not identity aliases.

21.16.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Procedure Booking Ingestion (p. 438)	feature FT	Canonical / Proposed
Feature: Procedural Documentation Ingestion (p. 439)	feature FT	Canonical / Proposed

21.16.11.8. Feature: Procedure Booking Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-21`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION.FEAT-SA-21`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration → BEC CT3: Procedure & Surgical Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FEAT-SA-21`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Procedure Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:324 (p. 1371)	Source standing does not approve the derived assertion.

21.16.11.8.1. Feature Overview

Procedure booking ingestion makes scheduled cases and procedural requests available in the wider care context. It conveys intended or arranged activity while retaining the distinction between a booked procedure and one actually performed.

21.16.11.8.2. Definition / governed semantic detail

Ingest scheduled surgical case bookings and procedural requests.

21.16.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.16.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Procedure Booking / Request (p. 440)	Established
From: associated feature	Service: Procedure Booking Ingress (p. 441)	Established

21.16.11.9. Feature: Procedural Documentation Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SA-22`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION.FEAT-SA-22`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration → BEC CT3: Procedure & Surgical Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FEAT-SA-22`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Procedure Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:325 (p. 1371)	Source standing does not approve the derived assertion.

21.16.11.9.1. Feature Overview

Procedural documentation ingestion preserves the operative, anaesthetic and post-procedure account supplied by the clinical service. It connects performed-care documentation with the wider record without giving the integration process authority to originate or judge the clinical findings.

21.16.11.9.2. Definition / governed semantic detail

Ingest operative reports, anesthesia records, and post-procedure summaries.

21.16.11.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.16.11.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Procedural Documentation (p. 442)	Established
From: associated feature	Service: Procedural Documentation Ingress (p. 443)	Established

21.16.12. Function: Receive Procedure Booking / Request

Property	Value	Meaning
Reference Id	`HCM-FN-receive-procedure-booking-request`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURE-BOOKING-REQUEST`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURE-BOOKING-REQUEST`	Direct to this identity; never reusable.
Evidence	sa:153 (p. 1445)	Source standing does not approve the derived assertion.

21.16.12.1. Definition / governed semantic detail

Ingests procedural and surgical requisitions.

21.16.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Procedure Booking Ingestion (p. 438)	Established

21.16.13. Service: Procedure Booking Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-procedure-booking-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURE-BOOKING-INGRESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURE-BOOKING-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:154 (p. 1445)	Source standing does not approve the derived assertion.

21.16.13.1. Definition / governed semantic detail

Receives surgical/procedural requests.

21.16.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Procedure Booking Ingestion (p. 438)	Established

21.16.14. Function: Receive Procedural Documentation

Property	Value	Meaning
Reference Id	`HCM-FN-receive-procedural-documentation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURAL-DOCUMENTATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURAL-DOCUMENTATION`	Direct to this identity; never reusable.
Evidence	sa:156 (p. 1445)	Source standing does not approve the derived assertion.

21.16.14.1. Definition / governed semantic detail

Ingests operation reports, anaesthetic records, and post-procedure notes.

21.16.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Procedural Documentation Ingestion (p. 439)	Established

21.16.15. Service: Procedural Documentation Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-procedural-documentation-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURAL-DOCUMENTATION-INGRESS`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Procedure Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Procedure Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURAL-DOCUMENTATION-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:157 (p. 1445)	Source standing does not approve the derived assertion.

21.16.15.1. Definition / governed semantic detail

Ingests procedural documentation, including surgical notes, into patient record streams.

21.16.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Procedural Documentation Ingestion (p. 439)	Established

21.17. BEC CT2: Clinical Record Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-record-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:249 (p. 1369); enabling:327 (p. 1371); sa:163 (p. 1445)	Source standing does not approve the derived assertion.

21.17.1. Capability Overview

Clinical Record Administration manages clinical documents as governed information with authorship, classification, versions and supersession history. It distinguishes a new document from an amendment or withdrawal and maintains metadata for discovery. Its purpose is to preserve an intelligible documentary record across change, rather than merely accumulate files.

21.17.2. Definition — Reviewed candidate

Clinical Record Administration manages clinical documents as governed information with authorship, classification, versions and supersession history. It distinguishes a new document from an amendment or withdrawal and maintains metadata for discovery. Its purpose is to preserve an intelligible documentary record across change, rather than merely accumulate files.

21.17.3. Boundary — retained v0.1 source explanation

This responsibility owns document governance; exchange of documents does not transfer that ownership to the transporting capability. Preservation does not originate the clinical facts.

21.17.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Non-destructive document lifecycle and governed discovery are direct platform information-management responsibilities.

21.17.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.17.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.17.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.17.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Receive Clinical Document (p. 450)	function	Canonical / Established
Service: Clinical Document Ingress (p. 450)	service	Canonical / Established
Function: Govern Clinical Document Lifecycle (p. 451)	function	Canonical / Established
Service: Clinical Document Lifecycle Service (p. 452)	service	Canonical / Established
Function: Index Clinical Document Metadata (p. 452)	function	Canonical / Established
Service: Document Metadata Registry Query (p. 453)	service	Canonical / Established
Process: Clinical Document Lifecycle Process (p. 454)	process	Canonical / Established
BEC CT3: Clinical Documentation Lifecycle Administration (p. 445)	capability CT3	Proposed / Proposed

21.17.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Information Management (p. 105)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed
To: enterprise enabled by	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed

21.17.10. BEC CT3: Clinical Documentation Lifecycle Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-documentation-lifecycle-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.17.10.1. Capability Overview

Administer ingestion, indexing, versioning, supersession and governed availability of clinical documentation. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.17.10.2. Definition — Reviewed candidate

Administer ingestion, indexing, versioning, supersession and governed availability of clinical documentation.

21.17.10.3. Intent

Administer ingestion, indexing, versioning, supersession and governed availability of clinical documentation.

21.17.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.17.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.17.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-0CE70156` (line 5625). Working locators are not identity aliases.

21.17.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Clinical Document Ingestion (p. 447)	feature FT	Canonical / Proposed
Feature: Document Versioning & Supersession (p. 448)	feature FT	Canonical / Proposed
Feature: Document Metadata Indexing (p. 449)	feature FT	Canonical / Proposed

21.17.10.8. Feature: Clinical Document Ingestion

Property	Value	Meaning
Reference Id	FEAT-SA-23	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-23	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration → BEC CT3: Clinical Documentation Lifecycle Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FEAT-SA-23	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Record Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:329 (p. 1372)	Source standing does not approve the derived assertion.

21.17.10.8.1. Feature Overview

Clinical document ingestion receives authored documents into governed information management. It supports both structured and unstructured contributions while preserving that receipt establishes a managed contribution, not independent clinical verification of every assertion in the document.

21.17.10.8.2. Definition / governed semantic detail

Ingest structured and unstructured clinical documents (discharge summaries, consult notes, letters).

21.17.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.17.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Clinical Document (p. 450)	Established
From: associated feature	Service: Clinical Document Ingress (p. 450)	Established

21.17.10.9. Feature: Document Versioning & Supersession

Property	Value	Meaning
Reference Id	FEAT-SA-24	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-24	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration → BEC CT3: Clinical Documentation Lifecycle Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FEAT-SA-24	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Record Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:330 (p. 1372)	Source standing does not approve the derived assertion.

21.17.10.9.1. Feature Overview

Versioning and supersession make document change understandable without destructive loss of the previous account. Author and amendment attribution distinguish who established each contribution; withdrawal or correction changes current standing without retrospectively erasing what Harmonia previously received or managed.

21.17.10.9.2. Definition / governed semantic detail

Manage document classification, author attribution, amendment attribution, versioning, and supersession or withdrawal without destructive data loss.

21.17.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.17.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Clinical Document Lifecycle (p. 451)	Established

Relationship	Related element	Assertion status
From: associated feature	Service: Clinical Document Lifecycle Service (p. 452)	Established

21.17.10.10. Feature: Document Metadata Indexing

Property	Value	Meaning
Reference Id	`FEAT-SA-25`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-25`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration → BEC CT3: Clinical Documentation Lifecycle Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FEAT-SA-25`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Record Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:331 (p. 1372)	Source standing does not approve the derived assertion.

21.17.10.10.1. Feature Overview

Metadata indexing makes documents discoverable by clinically and operationally relevant attributes. It describes content for search without making the index the document itself or granting access merely because a matching record can be located.

21.17.10.10.2. Definition / governed semantic detail

Index clinical document metadata (author, specialty, encounter, date, confidentiality) for rapid discovery.

21.17.10.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.17.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Index Clinical Document Metadata (p. 452)	Established
From: associated feature	Service: Document Metadata Registry Query (p. 453)	Established

21.17.11. Function: Receive Clinical Document

Property	Value	Meaning
Reference Id	`HCM-FN-receive-clinical-document`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-RECEIVE-CLINICAL-DOCUMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-RECEIVE-CLINICAL-DOCUMENT`	Direct to this identity; never reusable.
Evidence	sa:168 (p. 1445)	Source standing does not approve the derived assertion.

21.17.11.1. Definition / governed semantic detail

Receives clinical documents from information suppliers.

21.17.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinical Document Ingestion (p. 447)	Established

21.17.12. Service: Clinical Document Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-document-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-INGRESS`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:169 (p. 1445)	Source standing does not approve the derived assertion.

21.17.12.1. Definition / governed semantic detail

Exposes document submission endpoints across the HIE.

21.17.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Clinical Document Ingestion (p. 447)	Established

21.17.13. Function: Govern Clinical Document Lifecycle

Property	Value	Meaning
Reference Id	`HCM-FN-govern-clinical-document-lifecycle`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-GOVERN-CLINICAL-DOCUMENT-LIFECYCLE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-GOVERN-CLINICAL-DOCUMENT-LIFECYCLE`	Direct to this identity; never reusable.
Evidence	sa:171 (p. 1445); sa:174 (p. 1445)	Source standing does not approve the derived assertion.

21.17.13.1. Definition / governed semantic detail

Manages document states (**scoped illustration, not a universal transition model**): *Draft* → *Preliminary* → *Final* → *Amended* → *Superseded* → *Entered-in-Error*.

21.17.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Document Versioning & Supersession (p. 448)	Established
To: governed process	Process: Clinical Document Lifecycle Process (p. 454)	Established

21.17.14. Service: Clinical Document Lifecycle Service

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-document-lifecycle-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-LIFECYCLE-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-LIFECYCLE-SERVICE`	Direct to this identity; never reusable.
Evidence	sa:172 (p. 1445); sa:174 (p. 1445)	Source standing does not approve the derived assertion.

21.17.14.1. Definition / governed semantic detail

Allows authoring clinicians to publish addenda or corrections.

21.17.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Document Versioning & Supersession (p. 448)	Established

21.17.15. Function: Index Clinical Document Metadata

Property	Value	Meaning
Reference Id	`HCM-FN-index-clinical-document-metadata`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-INDEX-CLINICAL-DOCUMENT-METADATA`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-INDEX-CLINICAL-DOCUMENT-METADATA`	Direct to this identity; never reusable.
Evidence	sa:176 (p. 1445)	Source standing does not approve the derived assertion.

21.17.15.1. Definition / governed semantic detail

Indexes author, specialty, encounter, date, and document type (LOINC/SNOMED).

21.17.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Document Metadata Indexing (p. 449)	Established

21.17.16. Service: Document Metadata Registry Query

Property	Value	Meaning
Reference Id	`HCM-SV-document-metadata-registry-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-DOCUMENT-METADATA-REGISTRY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-DOCUMENT-METADATA-REGISTRY-QUERY`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	sa:177 (p. 1445)	Source standing does not approve the derived assertion.

21.17.16.1. Definition / governed semantic detail

Discloses document registry metadata to clinical viewers.

21.17.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Document Metadata Indexing (p. 449)	Established

21.17.17. Process: Clinical Document Lifecycle Process

Property	Value	Meaning
Reference Id	`HCM-PR-clinical-document-lifecycle-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.PR-CLINICAL-DOCUMENT-LIFECYCLE-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.PR-CLINICAL-DOCUMENT-LIFECYCLE-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:152 (p. 1417); processes:153 (p. 1418)	Source standing does not approve the derived assertion.

21.17.17.1. Process Overview

This Process makes authorship and changing document standing intelligible through the locally described review, amendment and withdrawal circumstances. It preserves prior versions rather than treating correction as erasure. The illustration does not require every document to pass through every stage or make finalisation, signing, legal qualification and originating authority equivalent.

21.17.17.2. Definition / governed semantic detail

- **Owning Capability**: `ClinicalRecordAdministration`

- **Process Purpose**: Governs the versioning, clinical sign-off, addenda, superseding, and legal status of clinical documents (discharge summaries, specialist letters, advance care directives).
- **State Progression Lifecycle**: Draft → Preliminary → Final Signed → Amended / Addended → Superseded → Entered-in-Error (Scoped illustration; no universal document lifecycle)
- **Key State Dispositions**:
  - `Draft`: Incomplete clinical document saved during consultation.
  - `Preliminary`: Document authored pending senior registrar or consultant countersignature.
  - `FinalSigned`: Authoritative clinical document legally signed and published.
  - `Amended/Addended`: Governed supplementary clinical addendum appended to signed document.
  - `Superseded`: Entire document replaced by a newer revision; prior version retained for audit.
  - `Entered-in-Error`: Document formally retracted due to wrong patient or invalid content; content struck through with retraction notice.
- **Scope qualification**: `Final` ≠ `automatically Final Signed`. The described signing/countersigning and legal qualifications remain local requirements, not a universal document lifecycle. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Amendment, supersession and entered-in-error are not mandatory stages for every document, and consumer processing does not establish originating authority or legal status.

21.17.17.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Govern Clinical Document Lifecycle (p. 451)	Established

21.18. BEC CT2: Care Plan Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-care-plan-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:250 (p. 1369); enabling:333 (p. 1372); sa:183 (p. 1446)	Source standing does not approve the derived assertion.

21.18.1. Capability Overview

Care Plan Administration receives care plans and coordinates the distribution of updates to authorised team members. It supports shared understanding of established clinical intent across participating organisations. The responsibility concerns the plan as managed information, distinct from deciding the intended course of care or performing the actions expressed by it.

21.18.2. Definition — Reviewed candidate

Care Plan Administration receives care plans and coordinates the distribution of updates to authorised team members. It supports shared understanding of established clinical intent across participating organisations. The responsibility concerns the plan as managed information, distinct from deciding the intended course of care or performing the actions expressed by it.

21.18.3. Boundary — retained v0.1 source explanation

Synchronising a plan does not author its clinical intent, approve treatment or prove planned care occurred.

21.18.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Timely propagation of multidisciplinary plan information materially supports continuity without transferring clinical planning authority.

21.18.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.18.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.18.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.18.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Community Care Coordination (p. 460)	capability CT3	Canonical / Proposed
BEC CT3: Outreach Care Continuity (p. 466)	capability CT3	Canonical / Proposed
Function: Receive Care Plan (p. 472)	function	Canonical / Established
Service: Care Plan Ingress (p. 472)	service	Canonical / Established
Function: Associate Care Plan Context (p. 473)	function	Canonical / Established
Service: Care Plan Context Query (p. 474)	service	Canonical / Established
Function: Distribute Care Plan Change (p. 474)	function	Canonical / Established
Service: Care Plan Change Notification (p. 475)	service	Canonical / Established

Element / page	Type / tier	Element / placement status
BEC CT3: Care Plan Lifecycle & Coordination (p. 457)	capability CT3	Proposed / Proposed

21.18.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT1: Individual Care Delivery (p. 5)	Proposed
From: enabled by	BC CT2: Care Coordination & Transition (p. 37)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

21.18.10. BEC CT3: Care Plan Lifecycle & Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-care-plan-lifecycle-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.18.10.1. Capability Overview

Administer received care plans, goals, revisions and cross-participant coordination while retaining clinical planning authority at its source. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.18.10.2. Definition — Reviewed candidate

Administer received care plans, goals, revisions and cross-participant coordination while retaining clinical planning authority at its source.

21.18.10.3. Intent

Administer received care plans, goals, revisions and cross-participant coordination while retaining clinical planning authority at its source.

21.18.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.18.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.18.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-E263E2D1` (line 5726). Working locators are not identity aliases.

21.18.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Care Plan Ingestion & Distribution (p. 458)	feature FT	Canonical / Proposed
Feature: Care Goal & Plan Progression Representation (p. 459)	feature FT	Proposed / Proposed

21.18.10.8. Feature: Care Plan Ingestion & Distribution

Property	Value	Meaning
Reference Id	`FEAT-SA-26`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION.FEAT-SA-26`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Care Plan Lifecycle & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FEAT-SA-26`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Care Plan Administration	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:335 (p. 1372)	Source standing does not approve the derived assertion.

21.18.10.8.1. Feature Overview

Care-plan exchange keeps authorised team members aware of established multidisciplinary intent and its updates. It provides information continuity while preserving clinical planning authority and the distinction between a distributed plan and care actually performed.

21.18.10.8.2. Definition / governed semantic detail

Ingest multidisciplinary care plans and syndicate updates to authorized care team members.

21.18.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.18.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Care Plan (p. 472)	Established
From: associated feature	Service: Care Plan Ingress (p. 472)	Established

21.18.10.9. Feature: Care Goal & Plan Progression Representation

Property	Value	Meaning
Reference Id	`HCM-FT-care-goal-plan-progression-representation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION.FEAT-STEP2-CARE-GOAL-PLAN-PROGRESSION-REPRESENTATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Care Plan Lifecycle & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FEAT-STEP2-CARE-GOAL-PLAN-PROGRESSION-REPRESENTATION`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

21.18.10.9.1. Feature Overview

Plan ingestion/distribution is complemented by the ability to distinguish the applicable goal and plan context as a shared plan progresses.

21.18.10.9.2. Definition — Proposed

Represent externally agreed care goals, participants and plan changes with their effective standing.

21.18.10.9.3. Boundary

Representing a goal does not choose treatment, determine clinical completion or grant plan approval authority.

21.18.10.9.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.18.10.9.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

21.18.10.9.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-D4E51095` (line 5752); `BEC-6CCF8E04` (line 5764). Working locators are not identity aliases.

21.18.11. BEC CT3: Community Care Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-community-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE`	Direct to this identity; never reusable.
Established name	Community Care	Current display rename is Proposed; meaning unchanged.
Name history	Community Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:374 (p. 1372); enabling:424 (p. 1374); sd:183 (p. 1453)	Source standing does not approve the derived assertion.

21.18.11.1. Capability Overview

Community Care integrates encounters, observations and plans across home, community and hospital outreach participants. It supports continuity where clinical activity occurs outside the hospital's immediate information environment. Receiving visit records and synchronising plan changes helps make community activity visible without turning the platform into the provider of that activity.

21.18.11.2. Boundary

Plan synchronisation does not originate clinical intent, assign community work or establish that a visit occurred beyond its source evidence.

21.18.11.3. Harmonia Architectural Relevance

Cross-organisational encounter and care-plan information materially supports community care continuity.

21.18.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.18.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.18.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.18.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Community Encounter Ingestion (p. 462)	feature FT	Canonical / Proposed
Feature: Community Care Plan Synchronization (p. 463)	feature FT	Canonical / Proposed
Function: Synchronize Community Visit Record (p. 463)	function	Canonical / Established
Service: Community Care Encounter Ingress (p. 464)	service	Canonical / Established
Function: Bind Community Support Network Context (p. 465)	function	Canonical / Established
Service: Community Care Team Query (p. 466)	service	Canonical / Established

21.18.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

21.18.11.9. Feature: Community Encounter Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SD-15`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-15`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Community Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-15`	Direct to this identity; never reusable.
Evidence	enabling:426 (p. 1374)	Source standing does not approve the derived assertion.

21.18.11.9.1. Feature Overview

Community encounter ingestion brings home-visit and community assessments into the longitudinal information context. It supports continuity across organisational boundaries while preserving source attribution and the difference between the documentation received and care performed by the platform.

21.18.11.9.2. Definition / governed semantic detail

Ingest home visit notes, community nursing observations, and social support assessments.

21.18.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Synchronize Community Visit Record (p. 463)	Established
From: associated feature	Service: Community Care Encounter Ingress (p. 464)	Established

21.18.11.10. Feature: Community Care Plan Synchronization

Property	Value	Meaning
Reference Id	`FEAT-SD-16`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-16`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Community Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-16`	Direct to this identity; never reusable.
Evidence	enabling:427 (p. 1374)	Source standing does not approve the derived assertion.

21.18.11.10.1. Feature Overview

Community plan synchronisation keeps hospital outreach and community participants informed of established plan changes. It supports shared clinical intent without allowing synchronisation to become authority to choose that intent or allocate clinical work.

21.18.11.10.2. Definition / governed semantic detail

Synchronize community care plan updates between hospital outreach teams and community health organizations.

21.18.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Bind Community Support Network Context (p. 465)	Unresolved association
From: associated feature	Service: Community Care Team Query (p. 466)	Unresolved association

21.18.11.11. Function: Synchronize Community Visit Record

Property	Value	Meaning
Reference Id	`HCM-FN-synchronize-community-visit-record`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FN-SYNCHRONIZE-COMMUNITY-VISIT-RECORD`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Community Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FN-SYNCHRONIZE-COMMUNITY-VISIT-RECORD`	Direct to this identity; never reusable.
Evidence	sd:187 (p. 1453)	Source standing does not approve the derived assertion.

21.18.11.11.1. Definition / governed semantic detail

Ingests clinical notes, wound assessments, and vital signs recorded in community and home settings.

21.18.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Community Encounter Ingestion (p. 462)	Established

21.18.11.12. Service: Community Care Encounter Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-community-care-encounter-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-ENCOUNTER-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Community Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-ENCOUNTER-INGRESS`	Direct to this identity; never reusable.
Evidence	sd:188 (p. 1453)	Source standing does not approve the derived assertion.

21.18.11.12.1. Definition / governed semantic detail

Ingests community care documentation.

21.18.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Community Encounter Ingestion (p. 462)	Established

21.18.11.13. Function: Bind Community Support Network Context

Property	Value	Meaning
Reference Id	`HCM-FN-bind-community-support-network-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FN-BIND-COMMUNITY-SUPPORT-NETWORK-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Community Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FN-BIND-COMMUNITY-SUPPORT-NETWORK-CONTEXT`	Direct to this identity; never reusable.
Evidence	sd:191 (p. 1453); sd:190 (p. 1453)	Source standing does not approve the derived assertion.

21.18.11.13.1. Definition / governed semantic detail

Links community nursing teams with local non-clinical social care and home care packages.

Feature association not established: Strategy `FEAT-SD-16 — Community Care Plan Synchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.18.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Community Care Plan Synchronization (p. 463)	Unresolved association

21.18.11.14. Service: Community Care Team Query

Property	Value	Meaning
Reference Id	`HCM-SV-community-care-team-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-TEAM-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Community Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-TEAM-QUERY`	Direct to this identity; never reusable.
Evidence	sd:192 (p. 1453); sd:190 (p. 1453)	Source standing does not approve the derived assertion.

21.18.11.14.1. Definition / governed semantic detail

Exposes community provider contact rosters.

Feature association not established: Strategy `FEAT-SD-16 — Community Care Plan Synchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

21.18.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Community Care Plan Synchronization (p. 463)	Unresolved association

21.18.12. BEC CT3: Outreach Care Continuity

Property	Value	Meaning
Reference Id	`HCM-BEC-outreach-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE`	Direct to this identity; never reusable.
Established name	Outreach Care	Current display rename is Proposed; meaning unchanged.
Name history	Outreach Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:375 (p. 1372); enabling:429 (p. 1374); sd:202 (p. 1453)	Source standing does not approve the derived assertion.

21.18.12.1. Capability Overview

Outreach Care supplies portable clinical context and receives documentation from rural or remote outreach activity. It addresses continuity where the relevant care information may not be available through the usual on-site environment. The responsibility connects the prior record with the outcomes reported after the visit while keeping source and temporal context intelligible.

21.18.12.2. Boundary

An offline-capable summary does not acquire clinical authority or guarantee that the remote copy represents current authoritative state.

21.18.12.3. Harmonia Architectural Relevance

Context provision and outcome ingestion materially connect outreach activity to the longitudinal record.

21.18.12.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.18.12.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.18.12.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.18.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Outreach Visit Context Provision (p. 468)	feature FT	Canonical / Proposed
Feature: Outreach Encounter Outcome Ingestion (p. 469)	feature FT	Canonical / Proposed
Function: Provide Outreach Visit Context (p. 470)	function	Canonical / Established
Function: Reconcile Outreach Clinical Batch (p. 470)	function	Canonical / Established
Service: Outreach Batch Reconciliation Service (p. 471)	service	Canonical / Established

21.18.12.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

21.18.12.9. Feature: Outreach Visit Context Provision

Property	Value	Meaning
Reference Id	`FEAT-SD-17`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-17`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Outreach Care Continuity	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-17`	Direct to this identity; never reusable.
Evidence	enabling:431 (p. 1375)	Source standing does not approve the derived assertion.

21.18.12.9.1. Feature Overview

Outreach context provision prepares portable summaries for care beyond the usual connected environment. Their usefulness depends on retaining clinical and temporal context; an offline-capable copy is not automatically current authoritative state or a new origin of clinical facts.

21.18.12.9.2. Definition / governed semantic detail

Package and deliver portable clinical summaries and offline-capable care summaries for remote outreach clinics.

21.18.12.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Provide Outreach Visit Context (p. 470)	Established

21.18.12.10. Feature: Outreach Encounter Outcome Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SD-18`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-18`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Outreach Care Continuity	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-18`	Direct to this identity; never reusable.
Evidence	enabling:432 (p. 1375)	Source standing does not approve the derived assertion.

21.18.12.10.1. Feature Overview

Outreach outcome ingestion brings documentation collected during remote visits back into the governed record. It reconnects previously separated activity with the regional context without erasing when, where and by whom the source information was established.

21.18.12.10.2. Definition / governed semantic detail

Ingest and reconcile clinical documentation gathered during rural and remote outreach visits.

21.18.12.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Reconcile Outreach Clinical Batch (p. 470)	Established
From: associated feature	Service: Outreach Batch Reconciliation Service (p. 471)	Established

21.18.12.11. Function: Provide Outreach Visit Context

Property	Value	Meaning
Reference Id	`HCM-FN-provide-outreach-visit-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FN-PROVIDE-OUTREACH-VISIT-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Outreach Care Continuity	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FN-PROVIDE-OUTREACH-VISIT-CONTEXT`	Direct to this identity; never reusable.
Evidence	sd:206 (p. 1453)	Source standing does not approve the derived assertion.

21.18.12.11.1. Definition / governed semantic detail

Packages and delivers portable clinical summaries and offline-capable care summaries for remote outreach clinics.

Derivation: `FEAT-SD-17` in Strategy's Outreach Care Continuity establishes this preparation/provision responsibility. Outcome reconciliation below does not supply pre-visit context. This Function remains owned by Outreach Care, uses governed source information without acquiring its originating authority and establishes no copying, storage or offline implementation mechanism.

21.18.12.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Outreach Visit Context Provision (p. 468)	Established

21.18.12.12. Function: Reconcile Outreach Clinical Batch

Property	Value	Meaning
Reference Id	`HCM-FN-reconcile-outreach-clinical-batch`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FN-RECONCILE-OUTREACH-CLINICAL-BATCH`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Outreach Care Continuity	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FN-RECONCILE-OUTREACH-CLINICAL-BATCH`	Direct to this identity; never reusable.
Evidence	sd:210 (p. 1453)	Source standing does not approve the derived assertion.

21.18.12.12.1. Definition / governed semantic detail

Reconciles and merges clinical documentation recorded during disconnected remote clinic operations.

21.18.12.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Outreach Encounter Outcome Ingestion (p. 469)	Established

21.18.12.13. Service: Outreach Batch Reconciliation Service

Property	Value	Meaning
Reference Id	`HCM-SV-outreach-batch-reconciliation-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.SV-OUTREACH-BATCH-RECONCILIATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration → BEC CT3: Outreach Care Continuity	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.SV-OUTREACH-BATCH-RECONCILIATION-SERVICE`	Direct to this identity; never reusable.
Evidence	sd:211 (p. 1453)	Source standing does not approve the derived assertion.

21.18.12.13.1. Definition / governed semantic detail

Reconciles batch clinical encounters upon network reconnection.

21.18.12.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Outreach Encounter Outcome Ingestion (p. 469)	Established

21.18.13. Function: Receive Care Plan

Property	Value	Meaning
Reference Id	`HCM-FN-receive-care-plan`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-RECEIVE-CARE-PLAN`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FN-RECEIVE-CARE-PLAN`	Direct to this identity; never reusable.
Evidence	sa:186 (p. 1446)	Source standing does not approve the derived assertion.

21.18.13.1. Definition / governed semantic detail

Ingests multidisciplinary care plans, clinical goals, and patient action steps.

21.18.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Care Plan Ingestion & Distribution (p. 458)	Established

21.18.14. Service: Care Plan Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-care-plan-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-INGRESS`	Direct to this identity; never reusable.
Evidence	sa:187 (p. 1446)	Source standing does not approve the derived assertion.

21.18.14.1. Definition / governed semantic detail

Ingests shared care plans.

21.18.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Care Plan Ingestion & Distribution (p. 458)	Established

21.18.15. Function: Associate Care Plan Context

Property	Value	Meaning
Reference Id	`HCM-FN-associate-care-plan-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-ASSOCIATE-CARE-PLAN-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FN-ASSOCIATE-CARE-PLAN-CONTEXT`	Direct to this identity; never reusable.
Evidence	sa:189 (p. 1446); sa:188 (p. 1446)	Source standing does not approve the derived assertion.

21.18.15.1. Definition / governed semantic detail

Links care plans to active diagnoses, health problems, and care team members.

21.18.16. Service: Care Plan Context Query

Property	Value	Meaning
Reference Id	`HCM-SV-care-plan-context-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CONTEXT-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CONTEXT-QUERY`	Direct to this identity; never reusable.
Evidence	sa:190 (p. 1446); sa:188 (p. 1446)	Source standing does not approve the derived assertion.

21.18.16.1. Definition / governed semantic detail

Discloses active care plans to treating clinicians.

21.18.17. Function: Distribute Care Plan Change

Property	Value	Meaning
Reference Id	`HCM-FN-distribute-care-plan-change`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-DISTRIBUTE-CARE-PLAN-CHANGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FN-DISTRIBUTE-CARE-PLAN-CHANGE`	Direct to this identity; never reusable.
Evidence	sa:192 (p. 1446); sa:191 (p. 1446)	Source standing does not approve the derived assertion.

21.18.17.1. Definition / governed semantic detail

Broadcasts care plan updates to all enrolled care team participants.

21.18.18. Service: Care Plan Change Notification

Property	Value	Meaning
Reference Id	`HCM-SV-care-plan-change-notification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CHANGE-NOTIFICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Care Plan Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CHANGE-NOTIFICATION`	Direct to this identity; never reusable.
Evidence	sa:193 (p. 1446); sa:191 (p. 1446)	Source standing does not approve the derived assertion.

21.18.18.1. Definition / governed semantic detail

Emits care plan revision notices.

21.19. BEC CT2: Clinical Communication Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-communication-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:251 (p. 1369); enabling:337 (p. 1372); sa:199 (p. 1446)	Source standing does not approve the derived assertion.

21.19.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.19.2. Capability Overview

This capability manages secure clinical communication transactions, delivery and receipt evidence, and the required enduring audit trail. Verified provider and team endpoints support appropriate addressing while transaction correlation makes the communication history intelligible. The responsibility preserves separate meanings for transport delivery, recipient acknowledgement and clinical or business disposition.

21.19.3. Definition — Reviewed candidate

This capability manages secure clinical communication transactions, delivery and receipt evidence, and the required enduring audit trail. Verified provider and team endpoints support appropriate addressing while transaction correlation makes the communication history intelligible. The responsibility preserves separate meanings for transport delivery, recipient acknowledgement and clinical or business disposition.

21.19.4. Boundary — retained v0.1 source explanation

Delivery or read receipts do not prove clinical review, incorporation, approval or acceptance. Audit evidence must remain governed without unmasked PHI in diagnostic streams.

21.19.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Secure addressed communication and attributable transaction evidence are substantive platform responsibilities.

21.19.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.19.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.19.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.19.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Distribute Clinical Communication (p. 482)	function	Canonical / Established
Service: Secure Clinical Message Dispatch (p. 483)	service	Canonical / Established
Function: Track Business Delivery Acknowledgement (p. 484)	function	Canonical / Established
Service: Communication Delivery Status Query (p. 485)	service	Canonical / Established
Function: Record Communication Transaction (p. 485)	function	Canonical / Established
Service: Communication Audit Query (p. 486)	service	Canonical / Established
BEC CT3: Secure Clinical Communication (p. 478)	capability CT3	Proposed / Proposed

21.19.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Digital Communication Channel (p. 138)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed
To: enterprise enabled by	Enterprise CT3: Governed Activity Audit Evidence (p. 872)	Proposed

21.19.11. BEC CT3: Secure Clinical Communication

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-secure-clinical-communication`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

21.19.11.1. Capability Overview

Administer the established secure clinical communication information and coordination responsibility while preserving its source authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

21.19.11.2. Definition — Proposed

Administer the established secure clinical communication information and coordination responsibility while preserving its source authority.

21.19.11.3. Intent

Administer the established secure clinical communication information and coordination responsibility while preserving its source authority.

21.19.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

21.19.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.19.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-6D84C2C1` (line 5799). Working locators are not identity aliases.

21.19.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Clinical Communication Distribution (p. 479)	feature FT	Canonical / Proposed
Feature: Delivery Acknowledgement Tracking (p. 480)	feature FT	Canonical / Proposed
Feature: Communication Audit Logging (p. 481)	feature FT	Canonical / Proposed

21.19.11.8. Feature: Clinical Communication Distribution

Property	Value	Meaning
Reference Id	`FEAT-SA-27`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-27`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration → BEC CT3: Secure Clinical Communication	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FEAT-SA-27`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Communication Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:339 (p. 1372)	Source standing does not approve the derived assertion.

21.19.11.8.1. Feature Overview

Clinical communication distribution connects verified providers and designated team endpoints through governed addressed delivery. The responsibility concerns reaching the intended participant, rather than assuming that delivered information has been read, incorporated or acted upon clinically.

21.19.11.8.2. Definition / governed semantic detail

Distribute secure clinical communications between verified healthcare providers and designated care team endpoints.

21.19.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.19.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Distribute Clinical Communication (p. 482)	Established
From: associated feature	Service: Secure Clinical Message Dispatch (p. 483)	Established

21.19.11.9. Feature: Delivery Acknowledgement Tracking

Property	Value	Meaning
Reference Id	`FEAT-SA-28`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-28`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration → BEC CT3: Secure Clinical Communication	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FEAT-SA-28`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Communication Administration	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:340 (p. 1372)	Source standing does not approve the derived assertion.

21.19.11.9.1. Feature Overview

Acknowledgement tracking associates delivery and read evidence with the communication transaction. Each receipt retains its contractual meaning; neither delivery nor reading alone proves business acceptance, clinical review, approval, finality or originating authority.

21.19.11.9.2. Definition / governed semantic detail

Correlate recipient delivery and read receipts with outbound clinical communications, distinguishing transport delivery from recipient acknowledgement. Receipt/read evidence does not by itself establish business acceptance, clinical review, incorporation, approval, finality or originating authority.

21.19.11.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.19.11.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Track Business Delivery Acknowledgement (p. 484)	Established
From: associated feature	Service: Communication Delivery Status Query (p. 485)	Established

21.19.11.10. Feature: Communication Audit Logging

Property	Value	Meaning
Reference Id	`FEAT-SA-29`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-29`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration → BEC CT3: Secure Clinical Communication	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FEAT-SA-29`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Communication Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:341 (p. 1372)	Source standing does not approve the derived assertion.

21.19.11.10.1. Feature Overview

Communication audit evidence preserves the attributable transaction history required for scrutiny of clinical communication. It records meaningful communication facts under the zero-PHI constraints rather than turning the audit trail into a copy of message content or a permanent diagnostic trace.

21.19.11.10.2. Definition / governed semantic detail

Maintain an enduring, non-PHI, tamper-evident log of all clinical communication transactions.

21.19.11.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

21.19.11.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Record Communication Transaction (p. 485)	Established
From: associated feature	Service: Communication Audit Query (p. 486)	Established

21.19.12. Function: Distribute Clinical Communication

Property	Value	Meaning
Reference Id	`HCM-FN-distribute-clinical-communication`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-DISTRIBUTE-CLINICAL-COMMUNICATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-DISTRIBUTE-CLINICAL-COMMUNICATION`	Direct to this identity; never reusable.
Evidence	sa:206 (p. 1446)	Source standing does not approve the derived assertion.

21.19.12.1. Definition / governed semantic detail

Routes secure messages, referral letters, and critical alerts to practitioner endpoints.

21.19.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinical Communication Distribution (p. 479)	Established

21.19.13. Service: Secure Clinical Message Dispatch

Property	Value	Meaning
Reference Id	`HCM-SV-secure-clinical-message-dispatch`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-SECURE-CLINICAL-MESSAGE-DISPATCH`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-SECURE-CLINICAL-MESSAGE-DISPATCH`	Direct to this identity; never reusable.
Evidence	sa:207 (p. 1446)	Source standing does not approve the derived assertion.

21.19.13.1. Definition / governed semantic detail

Exposes message dispatch to clinical authors.

21.19.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Clinical Communication Distribution (p. 479)	Established

21.19.14. Function: Track Business Delivery Acknowledgement

Property	Value	Meaning
Reference Id	`HCM-FN-track-business-delivery-acknowledgement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-TRACK-BUSINESS-DELIVERY-ACKNOWLEDGEMENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-TRACK-BUSINESS-DELIVERY-ACKNOWLEDGEMENT`	Direct to this identity; never reusable.
Evidence	sa:209 (p. 1446); sa:210 (p. 1446)	Source standing does not approve the derived assertion.

21.19.14.1. Definition / governed semantic detail

Tracks end-to-end delivery state and records business-level delivery/rejection acknowledgements.

21.19.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Delivery Acknowledgement Tracking (p. 480)	Established

21.19.15. Service: Communication Delivery Status Query

Property	Value	Meaning
Reference Id	`HCM-SV-communication-delivery-status-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-DELIVERY-STATUS-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-DELIVERY-STATUS-QUERY`	Direct to this identity; never reusable.
Evidence	sa:211 (p. 1446); sa:210 (p. 1446)	Source standing does not approve the derived assertion.

21.19.15.1. Definition / governed semantic detail

Discloses delivery confirmation to message senders.

21.19.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Delivery Acknowledgement Tracking (p. 480)	Established

21.19.16. Function: Record Communication Transaction

Property	Value	Meaning
Reference Id	`HCM-FN-record-communication-transaction`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-RECORD-COMMUNICATION-TRANSACTION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-RECORD-COMMUNICATION-TRANSACTION`	Direct to this identity; never reusable.
Evidence	sa:213 (p. 1446)	Source standing does not approve the derived assertion.

21.19.16.1. Definition / governed semantic detail

Maintains an immutable audit trail of clinical message dispatch and receipt.

21.19.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Communication Audit Logging (p. 481)	Established

21.19.17. Service: Communication Audit Query

Property	Value	Meaning
Reference Id	`HCM-SV-communication-audit-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-AUDIT-QUERY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Communication Administration; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement → BEC CT2: Clinical Communication Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-AUDIT-QUERY`	Direct to this identity; never reusable.
Evidence	sa:214 (p. 1446)	Source standing does not approve the derived assertion.

21.19.17.1. Definition / governed semantic detail

Discloses transmission history for medico-legal verification.

21.19.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Communication Audit Logging (p. 481)	Established

21.20. BEC CT2: Funding & Coverage Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-funding-coverage-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-FUNDING-COVERAGE-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FINANCIAL-INFORMATION-ENABLEMENT.CT3-BEC-FUNDING-COVERAGE-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:252 (p. 1369); enabling:343 (p. 1372)	Source standing does not approve the derived assertion.

21.20.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.20.2. Capability Overview

Funding & Coverage Administration represents externally supplied insurance coverage and eligibility flags needed as care context. It allows those administrative conditions to be available without implying that Harmonia determines entitlements. The established Adjacent standing and absence of Features bound the current responsibility to this information contribution.

21.20.3. Definition — Reviewed candidate

Funding & Coverage Administration represents externally supplied insurance coverage and eligibility flags needed as care context. It allows those administrative conditions to be available without implying that Harmonia determines entitlements. The established Adjacent standing and absence of Features bound the current responsibility to this information contribution.

21.20.4. Boundary — retained v0.1 source explanation

Coverage information is not claims adjudication, funding governance or a universal determination of care eligibility.

21.20.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Harmonia consumes the administrative context; it does not acquire the payer's authority or manage the underlying coverage arrangement.

21.20.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.20.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.20.8. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.20.9. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.20.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Financial & Commercial Services (p. 186)	Proposed

21.21. BEC CT2: Clinical Charging & Claims Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-charging-claims-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-CHARGING-CLAIMS-ADMINISTRATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FINANCIAL-INFORMATION-ENABLEMENT.CT3-BEC-CLINICAL-CHARGING-CLAIMS-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:253 (p. 1369); enabling:346 (p. 1372)	Source standing does not approve the derived assertion.

21.21.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

21.21.2. Capability Overview

This capability supplies clinical activity summaries and charging records to billing contexts. It preserves a route from activity information to the systems responsible for financial processing without treating the export as a completed bill or claim. No Feature decomposition or comprehensive revenue-cycle responsibility is established.

21.21.3. Definition — Reviewed candidate

This capability supplies clinical activity summaries and charging records to billing contexts. It preserves a route from activity information to the systems responsible for financial processing without treating the export as a completed bill or claim. No Feature decomposition or comprehensive revenue-cycle responsibility is established.

21.21.4. Boundary — retained v0.1 source explanation

Export of activity evidence does not confer pricing, billing, claim or accounting authority.

21.21.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Its Adjacent standing reflects information supplied to financial systems while the financial activity remains externally governed.

21.21.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

21.21.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

21.21.8. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

21.21.9. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

21.21.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Financial & Commercial Services (p. 186)	Proposed

22. BEC CT1: Clinical Service Dependency Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-service-dependency-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

22.2. Capability Overview

Identify and track operational and infrastructure dependencies necessary for clinical services to function, including care-place and environmental readiness, devices, materiel, forms, workstations, connectivity, infrastructure and basic working materials. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.3. Definition — Reviewed candidate

Identify and track operational and infrastructure dependencies necessary for clinical services to function, including care-place and environmental readiness, devices, materiel, forms, workstations, connectivity, infrastructure and basic working materials.

22.4. Intent

Identify and track operational and infrastructure dependencies necessary for clinical services to function, including care-place and environmental readiness, devices, materiel, forms, workstations, connectivity, infrastructure and basic working materials.

22.5. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.6. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Clinical Assessment (p. 492)	capability CT2	Canonical / Proposed
BEC CT2: Ambulatory Care (p. 498)	capability CT2	Canonical / Proposed
BEC CT2: Surgical & Procedural Care (p. 499)	capability CT2	Canonical / Proposed
BEC CT2: Therapeutic Services (p. 501)	capability CT2	Canonical / Proposed
BEC CT2: Allied Health (p. 504)	capability CT2	Canonical / Proposed
BEC CT2: Rehabilitation (p. 506)	capability CT2	Canonical / Proposed
BEC CT2: Primary Care (p. 494)	capability CT2	Proposed / Proposed
BEC CT2: Acute Care (p. 495)	capability CT2	Proposed / Proposed
BEC CT2: Emergency Care (p. 496)	capability CT2	Proposed / Proposed
BEC CT2: Inpatient Care (p. 497)	capability CT2	Proposed / Proposed
BEC CT2: Diagnostic Services (p. 500)	capability CT2	Proposed / Proposed
BEC CT2: Medication Therapy (p. 503)	capability CT2	Proposed / Proposed
BEC CT2: Preventive Care (p. 505)	capability CT2	Proposed / Proposed
BEC CT2: Community Care (p. 508)	capability CT2	Proposed / Proposed
BEC CT2: Outreach Care (p. 509)	capability CT2	Proposed / Proposed
BEC CT2: Remote & Virtual Care (p. 510)	capability CT2	Proposed / Proposed

22.8. BEC CT2: Clinical Assessment

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-CLINICAL-ASSESSMENT`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.

Property	Value	Meaning
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-CLINICAL-ASSESSMENT`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:361 (p. 1372)	Source standing does not approve the derived assertion.

22.8.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for clinical assessment services to function, retaining each dependency provider's responsibility.

22.8.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for clinical assessment services to function, retaining each dependency provider's responsibility.

22.8.3. Boundary — retained v0.1 source explanation

No atomic Features are established. The contextual name does not make Harmonia a clinical assessor or establish BC ancestry.

22.8.4. Harmonia Architectural Relevance — retained v0.1 source explanation

The Adjacent classification denotes contextual information participation rather than material execution of clinical assessment.

22.8.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

22.8.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

22.8.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

22.8.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.9. BEC CT2: Primary Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-primary-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-PRIMARY-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.9.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for primary care services to function, retaining each dependency provider's responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.9.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for primary care services to function, retaining each dependency provider's responsibility.

22.9.3. Intent

Identify and track the operational and infrastructure dependencies necessary for primary care services to function, retaining each dependency provider's responsibility.

22.9.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.9.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.10. BEC CT2: Acute Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-acute-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-ACUTE-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.10.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for acute care services to function, retaining each dependency provider's responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.10.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for acute care services to function, retaining each dependency provider's responsibility.

22.10.3. Intent

Identify and track the operational and infrastructure dependencies necessary for acute care services to function, retaining each dependency provider's responsibility.

22.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.11. BEC CT2: Emergency Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-emergency-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-EMERGENCY-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.11.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for emergency care services to function, retaining each dependency provider’s responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.11.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for emergency care services to function, retaining each dependency provider’s responsibility.

22.11.3. Intent

Identify and track the operational and infrastructure dependencies necessary for emergency care services to function, retaining each dependency provider’s responsibility.

22.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.12. BEC CT2: Inpatient Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-inpatient-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-INPATIENT-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.12.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for inpatient care services to function, retaining each dependency provider's responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.12.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for inpatient care services to function, retaining each dependency provider's responsibility.

22.12.3. Intent

Identify and track the operational and infrastructure dependencies necessary for inpatient care services to function, retaining each dependency provider's responsibility.

22.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.13. BEC CT2: Ambulatory Care

Property	Value	Meaning
Reference Id	`HCM-BEC-ambulatory-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-AMBULATORY-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-AMBULATORY-CARE`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:366 (p. 1372)	Source standing does not approve the derived assertion.

22.13.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for ambulatory care services to function, retaining each dependency provider’s responsibility.

22.13.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for ambulatory care services to function, retaining each dependency provider’s responsibility.

22.13.3. Boundary — retained v0.1 source explanation

The contextual entry does not allocate BC tiers or require a distinct platform function for every ambulatory activity.

22.13.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Adjacent standing preserves contextual participation while clinical service delivery remains external.

22.13.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

22.13.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

22.13.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

22.13.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.14. BEC CT2: Surgical & Procedural Care

Property	Value	Meaning
Reference Id	`HCM-BEC-surgical-procedural-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-SURGICAL-PROCEDURAL-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-SURGICAL-PROCEDURAL-CARE`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:367 (p. 1372)	Source standing does not approve the derived assertion.

22.14.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for surgical & procedural care services to function, retaining each dependency provider's responsibility.

22.14.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for surgical & procedural care services to function, retaining each dependency provider's responsibility.

22.14.3. Boundary — retained v0.1 source explanation

No atomic Features are allocated to this context, and Harmonia does not acquire surgical decision or procedural authority.

22.14.4. Harmonia Architectural Relevance — retained v0.1 source explanation

The context is represented without assigning direct clinical delivery responsibility.

22.14.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

22.14.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

22.14.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

22.14.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.15. BEC CT2: Diagnostic Services

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-diagnostic-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-DIAGNOSTIC-SERVICES`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.15.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for diagnostic services services to function, retaining each dependency provider's responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.15.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for diagnostic services services to function, retaining each dependency provider's responsibility.

22.15.3. Intent

Identify and track the operational and infrastructure dependencies necessary for diagnostic services services to function, retaining each dependency provider's responsibility.

22.15.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.15.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.16. BEC CT2: Therapeutic Services

Property	Value	Meaning
Reference Id	`HCM-BEC-therapeutic-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-THERAPEUTIC-SERVICES`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-THERAPEUTIC-SERVICES`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:369 (p. 1372)	Source standing does not approve the derived assertion.

22.16.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for therapeutic services services to function, retaining each dependency provider's responsibility.

22.16.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for therapeutic services services to function, retaining each dependency provider's responsibility.

22.16.3. Boundary — retained v0.1 source explanation

Information participation does not confer authority to select, perform or assess the clinical adequacy of therapy.

22.16.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Adjacent standing preserves the contextual role while treatment remains the responsible service's activity.

22.16.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

22.16.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

22.16.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

22.16.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.17. BEC CT2: Medication Therapy

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-medication-therapy`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-MEDICATION-THERAPY`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.17.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for medication therapy services to function, retaining each dependency provider’s responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.17.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for medication therapy services to function, retaining each dependency provider’s responsibility.

22.17.3. Intent

Identify and track the operational and infrastructure dependencies necessary for medication therapy services to function, retaining each dependency provider's responsibility.

22.17.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.17.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.18. BEC CT2: Allied Health

Property	Value	Meaning
Reference Id	`HCM-BEC-allied-health`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-ALLIED-HEALTH`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-ALLIED-HEALTH`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:371 (p. 1372)	Source standing does not approve the derived assertion.

22.18.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for allied health services to function, retaining each dependency provider's responsibility.

22.18.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for allied health services to function, retaining each dependency provider's responsibility.

22.18.3. Boundary — retained v0.1 source explanation

Care coordination and record access do not transfer professional judgement or therapy delivery to Harmonia.

22.18.4. Harmonia Architectural Relevance — retained v0.1 source explanation

The formal Adjacent position is retained rather than inherited from neighbouring Relevant delivery contexts.

22.18.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

22.18.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

22.18.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

22.18.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.19. BEC CT2: Preventive Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-preventive-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-PREVENTIVE-CARE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.19.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for preventive care services to function, retaining each dependency provider's responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.19.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for preventive care services to function, retaining each dependency provider's responsibility.

22.19.3. Intent

Identify and track the operational and infrastructure dependencies necessary for preventive care services to function, retaining each dependency provider's responsibility.

22.19.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.19.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.20. BEC CT2: Rehabilitation

Property	Value	Meaning
Reference Id	`HCM-BEC-rehabilitation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-REHABILITATION`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-REHABILITATION`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:373 (p. 1372)	Source standing does not approve the derived assertion.

22.20.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for rehabilitation services to function, retaining each dependency provider's responsibility.

22.20.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for rehabilitation services to function, retaining each dependency provider's responsibility.

22.20.3. Boundary — retained v0.1 source explanation

Information integration does not perform therapy, determine rehabilitation goals or judge clinical effectiveness.

22.20.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Contextual participation is retained within the formal Adjacent classification.

22.20.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

22.20.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

22.20.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

22.20.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.21. BEC CT2: Community Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-community-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-COMMUNITY-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.21.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for community care services to function, retaining each dependency provider's responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.21.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for community care services to function, retaining each dependency provider's responsibility.

22.21.3. Intent

Identify and track the operational and infrastructure dependencies necessary for community care services to function, retaining each dependency provider's responsibility.

22.21.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.21.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.22. BEC CT2: Outreach Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-outreach-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-OUTREACH-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.22.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for outreach care services to function, retaining each dependency provider’s responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.22.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for outreach care services to function, retaining each dependency provider’s responsibility.

22.22.3. Intent

Identify and track the operational and infrastructure dependencies necessary for outreach care services to function, retaining each dependency provider’s responsibility.

22.22.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.22.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

22.23. BEC CT2: Remote & Virtual Care

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-remote-virtual-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-REMOTE-VIRTUAL-CARE`	Current proposed context where marked Proposed.
Inherited release scope	**Not Addressed in R1.x / R2.x**	Applies through Clinical Service Dependency Management; independent of architectural status.
Current context	BEC CT1: Clinical Service Dependency Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

22.23.1. Capability Overview

Identify and track the operational and infrastructure dependencies necessary for remote & virtual care services to function, retaining each dependency provider’s responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

22.23.2. Definition — Reviewed candidate

Identify and track the operational and infrastructure dependencies necessary for remote & virtual care services to function, retaining each dependency provider’s responsibility.

22.23.3. Intent

Identify and track the operational and infrastructure dependencies necessary for remote & virtual care services to function, retaining each dependency provider’s responsibility.

22.23.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

22.23.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23. BEC CT1: Health Service Operations Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-operations-coordination-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT`	Current proposed context where marked Proposed.
Name history	Care Operations Coordination Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668	Source standing does not approve the derived assertion.

23.1. Capability Overview

Enable health-service operations through information about flow, care-places, capacity, workforce capability, operational work, logistics and discharge progression.

23.2. Definition — Reviewed candidate

Enable health-service operations through information about flow, care-places, capacity, workforce capability, operational work, logistics and discharge progression.

23.3. Boundary

Coordination owns only explicitly assigned operational activity. It does not originate clinical clearance, determine clinical work or take workforce and resource governance from their sources.

23.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668

23.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Represent and coordinate care-setting flow, operational resources, workforce participation, movement and service-delivery preparedness.

Previous overview: Operational coordination connects available resources with actual care-setting constraints and observable work progression. Bed state, local capacity, aggregate capacity, transport and discharge remain separate abilities even when they exchange the same milestones.

Previous intent: Make operational constraints and progression visible enough to coordinate the care-delivery responsibilities actually assigned to Harmonia.

23.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Clinic & Practice Operations (p. 514)	capability CT2	Canonical / Proposed
BEC CT2: Ward Operations (p. 524)	capability CT2	Canonical / Proposed
BEC CT2: Theatre Operations (p. 538)	capability CT2	Canonical / Proposed
BEC CT2: Emergency Department Operations (p. 548)	capability CT2	Canonical / Proposed
BEC CT2: Outpatient Operations (p. 561)	capability CT2	Canonical / Proposed
BEC CT2: Bed & Care-Place Management (p. 569)	capability CT2	Canonical / Proposed
BEC CT2: Service Capacity Management (p. 581)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Workforce Capability Monitoring (p. 587)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Rostering (p. 589)	capability CT2	Canonical / Proposed
BEC CT2: Mobile Staff Management (p. 591)	capability CT2	Canonical / Proposed
BEC CT2: On-Call Management (p. 601)	capability CT2	Canonical / Proposed
BEC CT2: Work Allocation & Dispatch (p. 607)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Training & Education (p. 618)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Competency Administration (p. 619)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Qualification Administration (p. 621)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Supervision (p. 630)	capability CT2	Canonical / Proposed
BEC CT2: Patient Transport (p. 632)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Logistics Coordination (p. 639)	capability CT2	Canonical / Proposed
BEC CT2: Discharge Management (p. 645)	capability CT2	Canonical / Proposed

Element / page	Type / tier	Element / placement status
BEC CT2: Service Establishment Support (p. 586)	capability CT2	Proposed / Proposed
BEC CT2: Credential Management (p. 627)	capability CT2	Proposed / Proposed

23.9. BEC CT2: Clinic & Practice Operations

Property	Value	Meaning
Reference Id	`HCM-BEC-clinic-practice-operations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:448 (p. 1375); enabling:481 (p. 1376); hso:72 (p. 1457)	Source standing does not approve the derived assertion.

23.9.1. Capability Overview

Clinic & Practice Operations tracks operating sessions and the operational progression of people through the clinic environment. It makes delays, waiting and completion context visible alongside service-specific capacity responsibilities. This is real-world clinic operation, distinct from the technical queueing machinery that might realise the information flow.

23.9.2. Definition — Reviewed candidate

Clinic & Practice Operations tracks operating sessions and the operational progression of people through the clinic environment. It makes delays, waiting and completion context visible alongside service-specific capacity responsibilities. This is real-world clinic operation, distinct from the technical queueing machinery that might realise the information flow.

23.9.3. Boundary — retained v0.1 source explanation

Operational queue progression does not determine clinical priority or own the clinician's worklist.

23.9.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Session and flow information materially supports clinic coordination while the clinic retains its contextual capacity responsibility.

23.9.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.9.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.9.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.9.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Clinic & Practice Capacity (p. 520)	function	Canonical / Established
Function: Track Clinic Operating Session (p. 520)	function	Canonical / Established
Function: Manage Clinic Queue Progression (p. 521)	function	Canonical / Established
Service: Clinic Queue Telemetry Service (p. 522)	service	Canonical / Established
Process: Clinic / Operational Queue Progression Process (p. 523)	process	Canonical / Established
BEC CT3: Session & Queue Coordination (p. 515)	capability CT3	Proposed / Proposed

23.9.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.9.10. BEC CT3: Session & Queue Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-session-queue-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.9.10.1. Capability Overview

Coordinate clinic/practice session state, operational queue progression and context-specific session capacity information. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.9.10.2. Definition — Reviewed candidate

Coordinate clinic/practice session state, operational queue progression and context-specific session capacity information.

23.9.10.3. Intent

Coordinate clinic/practice session state, operational queue progression and context-specific session capacity information.

23.9.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.9.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.9.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-FD9982FA` (line 7069). Working locators are not identity aliases.

23.9.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Clinic Session Tracking (p. 517)	feature FT	Canonical / Proposed
Feature: Operational Queue Progression (p. 518)	feature FT	Canonical / Proposed
Feature: Clinic Session Capacity Management (p. 519)	feature FT	Proposed / Proposed

23.9.10.8. Feature: Clinic Session Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-01`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-HSO-01`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations → BEC CT3: Session & Queue Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FEAT-HSO-01`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinic & Practice Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:483 (p. 1376)	Source standing does not approve the derived assertion.

23.9.10.8.1. Feature Overview

Clinic-session tracking records the operating session's start, delay and completion as service-level context. It is distinct from an individual patient's queue state: knowing where one patient is waiting does not establish whether the clinic session is running as planned.

23.9.10.8.2. Definition / governed semantic detail

Ingest and track clinic operating session start, delay, and completion states.

23.9.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.9.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Track Clinic Operating Session (p. 520)	Established

23.9.10.9. Feature: Operational Queue Progression

Property	Value	Meaning
Reference Id	`FEAT-HSO-02`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-HSO-02`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations → BEC CT3: Session & Queue Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FEAT-HSO-02`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinic & Practice Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:484 (p. 1376)	Source standing does not approve the derived assertion.

23.9.10.9.1. Feature Overview

Operational queue progression makes the patient's passage through clinic stages visible. The queue concerns real-world arrival, waiting and consultation context, rather than a broker queue, and does not give Harmonia authority to determine clinical priority or own the clinician's work-list.

23.9.10.9.2. Definition / governed semantic detail

Track client queue progression through reception, waiting area, consultation, and billing.

23.9.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.9.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Clinic Queue Progression (p. 521)	Established
From: associated feature	Service: Clinic Queue Telemetry Service (p. 522)	Established

23.9.10.10. Feature: Clinic Session Capacity Management

Property	Value	Meaning
Reference Id	`HCM-FT-clinic-session-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-STEP2-CLINIC-SESSION-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations → BEC CT3: Session & Queue Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FEAT-STEP2-CLINIC-SESSION-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

23.9.10.10.1. Feature Overview

Clinic capacity depends on session staffing and service mix as well as places; queue progression alone is insufficient.

23.9.10.10.2. Definition — Proposed

Represent available, committed and utilised session capacity under clinic-specific constraints.

23.9.10.10.3. Boundary

Local responsibility retains its capacity meaning; Service Capacity Management consumes it without acquiring ownership or imposing one common capacity model.

23.9.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.9.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

23.9.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Clinic & Practice Capacity (p. 520)	Proposed

23.9.11. Function: Manage Clinic & Practice Capacity

Property	Value	Meaning
Reference Id	`HCM-FN-manage-clinic-practice-capacity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-PRACTICE-CAPACITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-PRACTICE-CAPACITY`	Direct to this identity; never reusable.
Evidence	hso:75 (p. 1457); hso:76 (p. 1457)	Source standing does not approve the derived assertion.

23.9.11.1. Definition / governed semantic detail

Understands and manages available, committed and utilised clinic/practice capacity in relation to practitioner availability, sessions, rooms, appointments and patient demand.

Responsibility boundary: Operating-session tracking and individual queue progression contribute to this responsibility; neither alone expresses overall service capacity. Capacity coordination does not own source bookings, clinical decisions or practitioner rostering.

23.9.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinic Session Capacity Management (p. 519)	Proposed

23.9.12. Function: Track Clinic Operating Session

Property	Value	Meaning
Reference Id	`HCM-FN-track-clinic-operating-session`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-TRACK-CLINIC-OPERATING-SESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FN-TRACK-CLINIC-OPERATING-SESSION`	Direct to this identity; never reusable.
Evidence	hso:78 (p. 1457)	Source standing does not approve the derived assertion.

23.9.12.1. Definition / governed semantic detail

Ingests and tracks clinic operating-session start, delay and completion, distinct from an individual patient's queue progression.

Derivation: `FEAT-HSO-01` directly establishes this session-level behaviour; Manage Clinic Queue Progression and its Process concern individual patients and do not supply it.

23.9.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinic Session Tracking (p. 517)	Established

23.9.13. Function: Manage Clinic Queue Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-clinic-queue-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-QUEUE-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-QUEUE-PROGRESSION`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	hso:82 (p. 1457)	Source standing does not approve the derived assertion.

23.9.13.1. Definition / governed semantic detail

Tracks patient arrival, rooming, consultation in-progress, and check-out states.

23.9.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Operational Queue Progression (p. 518)	Established
To: governed process	Process: Clinic / Operational Queue Progression Process (p. 523)	Established

23.9.14. Service: Clinic Queue Telemetry Service

Property	Value	Meaning
Reference Id	`HCM-SV-clinic-queue-telemetry-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.SV-CLINIC-QUEUE-TELEMETRY-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.SV-CLINIC-QUEUE-TELEMETRY-SERVICE`	Direct to this identity; never reusable.
Evidence	hso:83 (p. 1457)	Source standing does not approve the derived assertion.

23.9.14.1. Definition / governed semantic detail

Discloses real-time clinic waiting times and queue states.

23.9.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Operational Queue Progression (p. 518)	Established

23.9.15. Process: Clinic / Operational Queue Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-clinic-operational-queue-progression-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.PR-CLINIC-OPERATIONAL-QUEUE-PROGRESSION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinic & Practice Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.PR-CLINIC-OPERATIONAL-QUEUE-PROGRESSION-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:187 (p. 1418); processes:188 (p. 1418)	Source standing does not approve the derived assertion.

23.9.15.1. Process Overview

This Process follows an individual's operational passage through the clinic. Waiting, rooming, consultation and departure have different implications for flow and communication, while the clinic's overall session has its own state. The progression is therefore patient-specific operational context rather than a complete account of clinic capacity or clinical priority.

23.9.15.2. Definition / governed semantic detail

- **Owning Capability**: `Clinic&PracticeOperations`
- **Process Purpose**: Governs the movement and state tracking of outpatients through an ambulatory specialty clinic session.
- **State Progression Lifecycle**: Scheduled → Patient Arrived → Checked-In → Roomed / Pre-Consult → Consultation In-Progress → Consultation Concluded → Departed
- **Key State Dispositions**:
  - `Scheduled`: Appointment booked on session list.
  - `PatientArrived`: Patient enters waiting area.
  - `Checked-In`: Reception verifies demographics and marks patient ready.
  - `Roomed/Pre-Consult`: Nursing staff record baseline vitals and room patient.
  - `ConsultationIn-Progress`: Clinician actively conducting consultation.
  - `ConsultationConcluded`: Clinical discussion finished; follow-up orders placed.
  - `Departed`: Patient checks out and departs clinic facility.

23.9.15.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Clinic Queue Progression (p. 521)	Established

23.10. BEC CT2: Ward Operations

Property	Value	Meaning
Reference Id	`HCM-BEC-ward-operations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:449 (p. 1375); enabling:486 (p. 1376); hso:90 (p. 1457)	Source standing does not approve the derived assertion.

23.10.1. Capability Overview

Ward Operations coordinates ward occupancy and care-place context and assembles the information needed around shift changes. Its responsibility includes the ward-specific meaning of capacity and patient flow rather than a generic resource counter. It can make pending concerns and tasks visible without becoming the authority managing clinical handover.

23.10.2. Definition — Reviewed candidate

Ward Operations coordinates ward occupancy and care-place context and assembles the information needed around shift changes. Its responsibility includes the ward-specific meaning of capacity and patient flow rather than a generic resource counter. It can make pending concerns and tasks visible without becoming the authority managing clinical handover.

23.10.3. Boundary — retained v0.1 source explanation

Handover information assembly is distinct from performing or managing handover. Context-specific capacity remains with Ward Operations.

23.10.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Correlated ward state and contextual information materially support operational continuity.

23.10.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.10.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.10.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.10.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	capability CT3	Canonical / Proposed
Function: Manage Ward Capacity (p. 536)	function	Canonical / Established
Service: Ward Operational Summary Query (p. 537)	service	Canonical / Established
Function: Assemble Ward Handover Context (p. 537)	function	Canonical / Established
BEC CT3: Ward Logistics Coordination (p. 525)	capability CT3	Proposed / Proposed

23.10.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.10.10. BEC CT3: Ward Logistics Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-ward-logistics-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.10.10.1. Capability Overview

Coordinate ward patient flow, handover context and ward-specific operational logistics and capacity information. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.10.10.2. Definition — Reviewed candidate

Coordinate ward patient flow, handover context and ward-specific operational logistics and capacity information.

23.10.10.3. Intent

Coordinate ward patient flow, handover context and ward-specific operational logistics and capacity information.

23.10.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.10.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.10.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-CE9BB804` (line 7179). Working locators are not identity aliases.

23.10.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Ward Patient Flow Coordination (p. 527)	feature FT	Canonical / Proposed

Element / page	Type / tier	Element / placement status
Feature: Clinical Handover Context Assembly (p. 528)	feature FT	Canonical / Proposed
Feature: Ward Care Capacity Management (p. 528)	feature FT	Proposed / Proposed

23.10.10.8. Feature: Ward Patient Flow Coordination

Property	Value	Meaning
Reference Id	FEAT-HSO-03	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-HSO-03	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Ward Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FEAT-HSO-03	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Ward Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:488 (p. 1376)	Source standing does not approve the derived assertion.

23.10.10.8.1. Feature Overview

Ward flow coordination connects occupancy, place allocation and bed status within the ward's operational context. Those facts contribute to ward capacity but do not exhaust its acuity, staffing and cohorting responsibilities or prove that another patient can be accepted.

23.10.10.8.2. Definition / governed semantic detail

Coordinate inpatient ward occupancy, care-place allocation, and patient bed status across care facilities.

23.10.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.10.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: contributes feature	Function: Manage Ward Capacity (p. 536)	Established

23.10.10.9. Feature: Clinical Handover Context Assembly

Property	Value	Meaning
Reference Id	`FEAT-HSO-04`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-HSO-04`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Ward Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FEAT-HSO-04`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Ward Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:489 (p. 1376)	Source standing does not approve the derived assertion.

23.10.10.9.1. Feature Overview

Handover context assembly brings patient lists, current concerns and pending tasks together for the participants conducting a shift handover. Assembly is the established responsibility: it does not perform clinical review, decide pending clinical work or manage the handover itself.

23.10.10.9.2. Definition / governed semantic detail

Collate current ward patient lists, active medical concerns, and pending tasks for nursing shift handovers.

23.10.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.10.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Assemble Ward Handover Context (p. 537)	Established

23.10.10.10. Feature: Ward Care Capacity Management

Property	Value	Meaning
Reference Id	`HCM-FT-ward-care-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-STEP2-WARD-CARE-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Ward Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FEAT-STEP2-WARD-CARE-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

23.10.10.10.1. Feature Overview

A clean bed can remain unusable when staffing or patient needs constrain the ward; source clinical facts remain attributed.

23.10.10.10.2. Definition — Proposed

Represent ward care capacity in relation to staffing, care-place suitability and patient dependency.

23.10.10.10.3. Boundary

Local responsibility retains its capacity meaning; Service Capacity Management consumes it without acquiring ownership or imposing one common capacity model.

23.10.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.10.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

23.10.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Ward Capacity (p. 536)	Proposed

23.10.11. BEC CT3: Multidisciplinary Inpatient Care Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-inpatient-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE`	Direct to this identity; never reusable.
Established name	Inpatient Care	Current display rename is Proposed; meaning unchanged.
Name history	Inpatient Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:365 (p. 1372); enabling:404 (p. 1374); sd:110 (p. 1451)	Source standing does not approve the derived assertion.

23.10.11.1. Capability Overview

Inpatient Care brings together information for multidisciplinary review and captures externally established clinical milestones during an admission. Results, medication records and nursing notes support a shared contextual view, while progression events make reported care status visible. The responsibility concerns information availability and correlation rather than control of the inpatient clinical worklist.

23.10.11.2. Boundary — v0.1 explanation

Clinical milestone capture does not establish the milestone's clinical authority or manage rounds and handover.

23.10.11.3. Harmonia Architectural Relevance

The platform materially supplies consolidated inpatient context; milestone tracking may be realised through approved reusable composition without an invented specialised Function.

23.10.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.10.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

23.10.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.10.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Inpatient Rounding Context Provision (p. 531)	feature FT	Canonical / Proposed
Feature: Inpatient Clinical Progression Tracking (p. 532)	feature FT	Canonical / Proposed
Function: Consolidate Inpatient Clinical Round Feed (p. 533)	function	Canonical / Established
Service: Ward Round Clinical Briefing Query (p. 534)	service	Canonical / Established
Function: Assemble Inpatient Discharge Dossier (p. 534)	function	Canonical / Established
Service: Discharge Dossier Packaging Service (p. 535)	service	Canonical / Established

23.10.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

23.10.11.9. Feature: Inpatient Rounding Context Provision

Property	Value	Meaning
Reference Id	`FEAT-SD-07`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FEAT-SD-07`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Multidisciplinary Inpatient Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FEAT-SD-07`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	enabling:406 (p. 1374)	Source standing does not approve the derived assertion.

23.10.11.9.1. Feature Overview

Rounding context brings results, medication records and nursing notes into an accessible multidisciplinary view. Assembly serves the clinical discussion but does not perform rounds, determine pending clinical work or establish the team's professional conclusions.

23.10.11.9.2. Definition / governed semantic detail

Aggregate diagnostic results, current medication administration records, and nursing notes for multidisciplinary inpatient rounds.

23.10.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Consolidate Inpatient Clinical Round Feed (p. 533)	Established
From: associated feature	Service: Ward Round Clinical Briefing Query (p. 534)	Established

23.10.11.10. Feature: Inpatient Clinical Progression Tracking

Property	Value	Meaning
Reference Id	`FEAT-SD-08`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FEAT-SD-08`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Multidisciplinary Inpatient Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FEAT-SD-08`	Direct to this identity; never reusable.
Evidence	enabling:407 (p. 1374)	Source standing does not approve the derived assertion.

23.10.11.10.1. Feature Overview

Inpatient progression tracking captures clinical milestones reported by the responsible participants. General clinical-information capabilities and configured workflow composition suffice for the established Feature; capture does not require inventing a specialised Function or make Harmonia the authority declaring clinical stability.

23.10.11.10.2. Definition / governed semantic detail

Capture inpatient clinical milestone completions (e.g., medical stability achieved, allied health clearance).

23.10.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Assemble Inpatient Discharge Dossier (p. 534)	Unresolved association
From: associated feature	Service: Discharge Dossier Packaging Service (p. 535)	Unresolved association

23.10.11.11. Function: Consolidate Inpatient Clinical Round Feed

Property	Value	Meaning
Reference Id	`HCM-FN-consolidate-inpatient-clinical-round-feed`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FN-CONSOLIDATE-INPATIENT-CLINICAL-ROUND-FEED`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Multidisciplinary Inpatient Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FN-CONSOLIDATE-INPATIENT-CLINICAL-ROUND-FEED`	Direct to this identity; never reusable.
Evidence	sd:114 (p. 1451)	Source standing does not approve the derived assertion.

23.10.11.11.1. Definition / governed semantic detail

Aggregates daily vital signs, overnight pathology, allied health notes, and medication administration records.

23.10.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Inpatient Rounding Context Provision (p. 531)	Established

23.10.11.12. Service: Ward Round Clinical Briefing Query

Property	Value	Meaning
Reference Id	`HCM-SV-ward-round-clinical-briefing-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.SV-WARD-ROUND-CLINICAL-BRIEFING-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Multidisciplinary Inpatient Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.SV-WARD-ROUND-CLINICAL-BRIEFING-QUERY`	Direct to this identity; never reusable.
Evidence	sd:115 (p. 1451)	Source standing does not approve the derived assertion.

23.10.11.12.1. Definition / governed semantic detail

Discloses consolidated 24-hour clinical briefing views.

23.10.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Inpatient Rounding Context Provision (p. 531)	Established

23.10.11.13. Function: Assemble Inpatient Discharge Dossier

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-inpatient-discharge-dossier`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FN-ASSEMBLE-INPATIENT-DISCHARGE-DOSSIER`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Multidisciplinary Inpatient Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FN-ASSEMBLE-INPATIENT-DISCHARGE-DOSSIER`	Direct to this identity; never reusable.
Evidence	sd:118 (p. 1451); sd:117 (p. 1451)	Source standing does not approve the derived assertion.

23.10.11.13.1. Definition / governed semantic detail

Synthesises inpatient diagnoses, procedural summaries, reconciled discharge medications, and follow-up instructions into a canonical discharge summary package.

Feature association not established: `FEAT-SD-08 — Inpatient Clinical Progression Tracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.

23.10.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Inpatient Clinical Progression Tracking (p. 532)	Unresolved association

23.10.11.14. Service: Discharge Dossier Packaging Service

Property	Value	Meaning
Reference Id	`HCM-SV-discharge-dossier-packaging-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.SV-DISCHARGE-DOSSIER-PACKAGING-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations → BEC CT3: Multidisciplinary Inpatient Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.SV-DISCHARGE-DOSSIER-PACKAGING-SERVICE`	Direct to this identity; never reusable.
Evidence	sd:119 (p. 1451); sd:117 (p. 1451)	Source standing does not approve the derived assertion.

23.10.11.14.1. Definition / governed semantic detail

Emits canonical discharge summaries to GPs and community providers.

Feature association not established: `FEAT-SD-08 — Inpatient Clinical Progression Tracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.

23.10.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Inpatient Clinical Progression Tracking (p. 532)	Unresolved association

23.10.12. Function: Manage Ward Capacity

Property	Value	Meaning
Reference Id	`HCM-FN-manage-ward-capacity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.FN-MANAGE-WARD-CAPACITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FN-MANAGE-WARD-CAPACITY`	Direct to this identity; never reusable.
Name history	Coordinate Ward Operational State	Names do not establish another identity.
Evidence	hso:93 (p. 1457); hso:94 (p. 1457); hso:97 (p. 1457)	Source standing does not approve the derived assertion.

23.10.12.1. Definition / governed semantic detail

Coordinates available, committed and utilised ward capacity and its patient-flow implications, retaining acuity distribution, nurse-to-patient ratios and isolation cohorting alongside bed/care-place state, patient demand and operational constraints.

Capacity boundary: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.

23.10.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: contributes feature	Feature: Ward Patient Flow Coordination (p. 527)	Established

Relationship	Related element	Assertion status
To: realises feature	Feature: Ward Care Capacity Management (p. 528)	Proposed

23.10.13. Service: Ward Operational Summary Query

Property	Value	Meaning
Reference Id	`HCM-SV-ward-operational-summary-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.SV-WARD-OPERATIONAL-SUMMARY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.SV-WARD-OPERATIONAL-SUMMARY-QUERY`	Direct to this identity; never reusable.
Evidence	hso:95 (p. 1457); hso:97 (p. 1457)	Source standing does not approve the derived assertion.

23.10.13.1. Definition / governed semantic detail

Discloses ward-level operational telemetry.

Capacity boundary: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.

23.10.14. Function: Assemble Ward Handover Context

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-ward-handover-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.FN-ASSEMBLE-WARD-HANDOVER-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Ward Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FN-ASSEMBLE-WARD-HANDOVER-CONTEXT`	Direct to this identity; never reusable.
Evidence	hso:99 (p. 1457)	Source standing does not approve the derived assertion.

23.10.14.1. Definition / governed semantic detail

Collates current ward patient lists, active medical concerns and pending tasks for nursing shift handover.

Derivation: `FEAT-HSO-04` directly requires this assembly. Ward acuity/cohorting telemetry alone does not provide the stated handover content. Source clinical information retains its originating authority; assembly does not perform clinical review, manage clinical handover, determine pending clinical work or acquire clinical judgement.

23.10.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinical Handover Context Assembly (p. 528)	Established

23.11. BEC CT2: Theatre Operations

Property	Value	Meaning
Reference Id	`HCM-BEC-theatre-operations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS`	Direct to this identity; never reusable.

Property	Value	Meaning
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:450 (p. 1375); enabling:491 (p. 1376); hso:106 (p. 1458)	Source standing does not approve the derived assertion.

23.11.1. Capability Overview

Theatre Operations integrates surgical case progression and the operational consequences relevant to portering, sterilisation and recovery participants. It gives reported milestones an operational coordination context without originating the surgical acts they describe. Theatre-specific capacity remains a business responsibility even when shared information machinery supports several service contexts.

23.11.2. Definition — Reviewed candidate

Theatre Operations integrates surgical case progression and the operational consequences relevant to portering, sterilisation and recovery participants. It gives reported milestones an operational coordination context without originating the surgical acts they describe. Theatre-specific capacity remains a business responsibility even when shared information machinery supports several service contexts.

23.11.3. Boundary — retained v0.1 source explanation

Tracking incision or recovery information does not authorise surgery or transfer control of the clinical procedure.

23.11.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Milestone-driven operational notifications materially connect supporting activity around the theatre service.

23.11.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.11.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.11.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.11.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Theatre Capacity (p. 544)	function	Canonical / Established
Function: Manage Theatre Case Progression (p. 545)	function	Canonical / Established
Service: Theatre Case Progression Telemetry (p. 546)	service	Canonical / Established
Process: Theatre Case Progression Process (p. 547)	process	Canonical / Established
BEC CT3: Perioperative Logistics Coordination (p. 540)	capability CT3	Proposed / Proposed

23.11.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.11.10. BEC CT3: Perioperative Logistics Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-perioperative-logistics-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.11.10.1. Capability Overview

Coordinate theatre case progression information and notifications to supporting perioperative operational teams. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.11.10.2. Definition — Reviewed candidate

Coordinate theatre case progression information and notifications to supporting perioperative operational teams.

23.11.10.3. Intent

Coordinate theatre case progression information and notifications to supporting perioperative operational teams.

23.11.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.11.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.11.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-D248D868` (line 7287). Working locators are not identity aliases.

23.11.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Operating Theatre Case Progression (p. 541)	feature FT	Canonical / Proposed
Feature: Perioperative Resource Notification (p. 542)	feature FT	Canonical / Proposed
Feature: Theatre Session Capacity Management (p. 543)	feature FT	Proposed / Proposed

23.11.10.8. Feature: Operating Theatre Case Progression

Property	Value	Meaning
Reference Id	`FEAT-HSO-05`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-HSO-05`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations → BEC CT3: Perioperative Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FEAT-HSO-05`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Theatre Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:493 (p. 1376)	Source standing does not approve the derived assertion.

23.11.10.8.1. Feature Overview

Case progression records externally established surgical milestones in an operationally meaningful sequence. It makes the theatre situation visible without authorising or performing the clinical acts denoted by anaesthesia, incision, closure or recovery.

23.11.10.8.2. Definition / governed semantic detail

Track surgical case state transitions (`CALLED` → `IN_THEATRE` → `ANAESTHETISED` → `INCISION` → `CLOSED` → `RECOVERY`).

23.11.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.11.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Theatre Case Progression (p. 545)	Established
From: associated feature	Service: Theatre Case Progression Telemetry (p. 546)	Established

23.11.10.9. Feature: Perioperative Resource Notification

Property	Value	Meaning
Reference Id	`FEAT-HSO-06`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-HSO-06`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations → BEC CT3: Perioperative Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FEAT-HSO-06`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Theatre Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:494 (p. 1376)	Source standing does not approve the derived assertion.

23.11.10.9.1. Feature Overview

Perioperative notification connects relevant case milestones with portering, sterilisation and recovery participants. Observation alone does not realise the Feature's dispatch responsibility; the established case-progression behaviour includes the required alerts without adding another Function or Service.

23.11.10.9.2. Definition / governed semantic detail

Dispatch alerts to portering, sterilization, and recovery teams based on surgical case progression milestones.

23.11.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.11.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Theatre Case Progression (p. 545)	Established

23.11.10.10. Feature: Theatre Session Capacity Management

Property	Value	Meaning
Reference Id	`HCM-FT-theatre-session-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-STEP2-THEATRE-SESSION-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations → BEC CT3: Perioperative Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FEAT-STEP2-THEATRE-SESSION-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

23.11.10.10.1. Feature Overview

Theatre capacity involves session time, team capability, equipment and recovery provision rather than a count of theatres.

23.11.10.10.2. Definition — Proposed

Represent available and committed procedural-session capacity with its resource dependencies.

23.11.10.10.3. Boundary

Local responsibility retains its capacity meaning; Service Capacity Management consumes it without acquiring ownership or imposing one common capacity model.

23.11.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.11.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

23.11.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Theatre Capacity (p. 544)	Proposed

23.11.11. Function: Manage Theatre Capacity

Property	Value	Meaning
Reference Id	`HCM-FN-manage-theatre-capacity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CAPACITY`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CAPACITY`	Direct to this identity; never reusable.
Evidence	hso:109 (p. 1458); hso:110 (p. 1458)	Source standing does not approve the derived assertion.

23.11.11.1. Definition / governed semantic detail

Understands and manages available, committed and utilised theatre capacity in relation to theatre, team and equipment availability, scheduled cases and case progression.

Responsibility boundary: Case milestones and targeted resource notifications contribute; they do not by themselves establish available or committed theatre service capacity. Clinical case determination and originating team/equipment information remain separately governed.

23.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Theatre Session Capacity Management (p. 543)	Proposed

23.11.12. Function: Manage Theatre Case Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-theatre-case-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CASE-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CASE-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:112 (p. 1458)	Source standing does not approve the derived assertion.

23.11.12.1. Definition / governed semantic detail

Coordinates surgical case milestones: *Case Called → Patient in Anaesthetic Bay → Anaesthesia Commenced → Knife to Skin → Procedure Finished → In PACU/Recovery → Ward Handover Complete*.

Additional Feature responsibility: `FEAT-HSO-06 — Perioperative Resource Notification` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.

23.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Operating Theatre Case Progression (p. 541)	Established
To: realises feature	Feature: Perioperative Resource Notification (p. 542)	Established
To: governed process	Process: Theatre Case Progression Process (p. 547)	Established

23.11.13. Service: Theatre Case Progression Telemetry

Property	Value	Meaning
Reference Id	`HCM-SV-theatre-case-progression-telemetry`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.SV-THEATRE-CASE-PROGRESSION-TELEMETRY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.SV-THEATRE-CASE-PROGRESSION-TELEMETRY`	Direct to this identity; never reusable.
Evidence	hso:113 (p. 1458)	Source standing does not approve the derived assertion.

23.11.13.1. Definition / governed semantic detail

Emits real-time surgical milestone updates.

Additional Feature responsibility: `FEAT-HSO-06 — Perioperative Resource Notification` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.

23.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Operating Theatre Case Progression (p. 541)	Established

23.11.14. Process: Theatre Case Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-theatre-case-progression-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.PR-THEATRE-CASE-PROGRESSION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Theatre Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.PR-THEATRE-CASE-PROGRESSION-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:170 (p. 1418); processes:171 (p. 1418)	Source standing does not approve the derived assertion.

23.11.14.1. Process Overview

Theatre case progression follows reported milestones of the surgical encounter so that supporting participants can coordinate around the actual situation. The state account concerns what the clinical service reports has occurred. It does not make the integration mechanism the actor administering anaesthesia, performing surgery or managing clinical handover.

23.11.14.2. Definition / governed semantic detail

- **Owning Capability**: `TheatreOperations`
- **Process Purpose**: Governs the perioperative operational milestones of a surgical procedure within the operating theatre suite.
- **State Progression Lifecycle**: Case Called → In Anaesthetic Bay → Anaesthesia Commenced → Patient in Theatre → Knife-to-Skin → Procedure Finished → In PACU Recovery → Ward Handover Complete
- **Key State Dispositions**:
  - `CaseCalled`: Theatre team requests ward to send patient for surgery.
  - `InAnaestheticBay`: Patient arrives in theatre holding and identity/consent verified.
  - `AnaesthesiaCommenced`: Anaesthetic induction underway.
  - `PatientinTheatre`: Patient transferred onto operating table.
  - `Knife-to-Skin`: Surgical incision initiated (operative start).
  - `ProcedureFinished`: Wound closure completed and dressings applied.

- `InPACURecovery`: Patient monitored in post-anaesthesia care unit.
- `WardHandoverComplete`: Post-operative clinical handover to inpatient ward completed.

23.11.14.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Theatre Case Progression (p. 545)	Established

23.12. BEC CT2: Emergency Department Operations

Property	Value	Meaning
Reference Id	`HCM-BEC-emergency-department-operations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:451 (p. 1375); enabling:496 (p. 1376); hso:121 (p. 1458)	Source standing does not approve the derived assertion.

23.12.1. Capability Overview

This capability maintains the operational context of emergency places, waits and length-of-stay milestones. It supports understanding of occupancy and impediments in the department while keeping capacity's emergency-service meaning intact. Its concern differs from Emergency Care's clinical presentation context, even though both can refer to the same patient encounter.

23.12.2. Definition — Reviewed candidate

This capability maintains the operational context of emergency places, waits and length-of-stay milestones. It supports understanding of occupancy and impediments in the department while keeping capacity's emergency-service meaning intact. Its concern differs from Emergency Care's clinical presentation context, even though both can refer to the same patient encounter.

23.12.3. Boundary — retained v0.1 source explanation

Operational timing and escalation do not determine triage or acquire clinical decision authority.

23.12.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Care-place state and operational waiting information materially support coordination of the emergency service.

23.12.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.12.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.12.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.12.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Emergency Care Progression (p. 554)	capability CT3	Canonical / Proposed
Function: Coordinate ED Departmental Capacity (p. 560)	function	Canonical / Established
Service: ED Operational Status Query (p. 561)	service	Canonical / Established
BEC CT3: Emergency Department Operational Coordination (p. 549)	capability CT3	Proposed / Proposed

23.12.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.12.10. BEC CT3: Emergency Department Operational Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-emergency-department-operational-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.12.10.1. Capability Overview

Coordinate emergency-department bed/bay occupancy, operational waits, length-of-stay milestones and local capacity information. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.12.10.2. Definition — Reviewed candidate

Coordinate emergency-department bed/bay occupancy, operational waits, length-of-stay milestones and local capacity information.

23.12.10.3. Intent

Coordinate emergency-department bed/bay occupancy, operational waits, length-of-stay milestones and local capacity information.

23.12.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.12.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.12.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-469419AB` (line 7376). Working locators are not identity aliases.

23.12.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: ED Bed & Bay Tracking (p. 551)	feature FT	Canonical / Proposed
Feature: ED Length of Stay Tracking (p. 552)	feature FT	Canonical / Proposed
Feature: Emergency Treatment Capacity Management (p. 553)	feature FT	Proposed / Proposed

23.12.10.8. Feature: ED Bed & Bay Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-07`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-HSO-07`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Department Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FEAT-HSO-07`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Emergency Department Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:498 (p. 1376)	Source standing does not approve the derived assertion.

23.12.10.8.1. Feature Overview

Emergency-place tracking distinguishes occupancy and status across cubicles, resuscitation bays and short-stay beds. It provides place context for departmental operations while leaving the service's usable capacity and clinical triage decisions separate from an inventory of spaces.

23.12.10.8.2. Definition / governed semantic detail

Track occupancy and status of emergency cubicles, resuscitation bays, and short-stay beds.

23.12.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.12.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Coordinate ED Departmental Capacity (p. 560)	Established

23.12.10.9. Feature: ED Length of Stay Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-08`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-HSO-08`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Department Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FEAT-HSO-08`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Emergency Department Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:499 (p. 1376)	Source standing does not approve the derived assertion.

23.12.10.9.1. Feature Overview

Length-of-stay tracking makes waits and operational timing thresholds visible for escalation. The timing information supports management of impediments without determining clinical urgency or making an elapsed-time alert an independent assurance finding.

23.12.10.9.2. Definition / governed semantic detail

Monitor operational wait times and length-of-stay milestones, generating operational escalation notifications.

23.12.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.12.10.10. Feature: Emergency Treatment Capacity Management

Property	Value	Meaning
Reference Id	`HCM-FT-emergency-treatment-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-STEP2-EMERGENCY-TREATMENT-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Department Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FEAT-STEP2-EMERGENCY-TREATMENT-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

23.12.10.10.1. Feature Overview

Bay state and length of stay contribute evidence but do not alone establish the ability to accept another care demand.

23.12.10.10.2. Definition — Proposed

Represent current emergency treatment capacity under space, staffing and acuity constraints.

23.12.10.10.3. Boundary

Local responsibility retains its capacity meaning; Service Capacity Management consumes it without acquiring ownership or imposing one common capacity model.

23.12.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.12.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

23.12.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Coordinate ED Departmental Capacity (p. 560)	Proposed

23.12.11. BEC CT3: Emergency Care Progression

Property	Value	Meaning
Reference Id	`HCM-BEC-emergency-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE`	Direct to this identity; never reusable.
Established name	Emergency Care	Current display rename is Proposed; meaning unchanged.
Name history	Emergency Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:364 (p. 1372); enabling:399 (p. 1373); sd:91 (p. 1450)	Source standing does not approve the derived assertion.

23.12.11.1. Capability Overview

Emergency Care integrates presentation and triage information and correlates it with recent admissions, summaries and known allergies. It supports a rapidly intelligible account of the patient arriving for emergency care without replacing the clinical triage decision. The context is valuable because the regional history may be distributed across participants outside the receiving department.

23.12.11.2. Boundary

Ingested triage categories remain externally established clinical information; correlation does not confer emergency decision authority.

23.12.11.3. Harmonia Architectural Relevance

Cross-facility context and current presentation information materially support emergency care continuity.

23.12.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.12.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

23.12.11.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.12.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	feature FT	Canonical / Proposed
Feature: Emergency Care Cross-Facility Correlation (p. 556)	feature FT	Canonical / Proposed
Function: Bind ED Triage Assessment (p. 557)	function	Canonical / Established
Service: ED Triage Stream Ingress (p. 558)	service	Canonical / Established
Function: Provide Emergency LHR Summary (p. 558)	function	Canonical / Established
Service: Emergency Fast-Path LHR Query (p. 559)	service	Canonical / Established

23.12.11.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

23.12.11.9. Feature: Emergency Department Arrival & Triage Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SD-05`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FEAT-SD-05`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Care Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FEAT-SD-05`	Direct to this identity; never reusable.
Evidence	enabling:401 (p. 1374)	Source standing does not approve the derived assertion.

23.12.11.9.1. Feature Overview

Emergency presentation ingestion retains the arrival, triage category and presenting complaint reported by the emergency context. It makes that event available for regional correlation while preserving that the clinical triage judgement was established by the responsible source.

23.12.11.9.2. Definition / governed semantic detail

Ingest emergency presentation, triage category, and chief complaint records.

23.12.11.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Bind ED Triage Assessment (p. 557)	Established
From: associated feature	Service: ED Triage Stream Ingress (p. 558)	Established

23.12.11.10. Feature: Emergency Care Cross-Facility Correlation

Property	Value	Meaning
Reference Id	`FEAT-SD-06`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FEAT-SD-06`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Care Progression	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FEAT-SD-06`	Direct to this identity; never reusable.
Evidence	enabling:402 (p. 1374)	Source standing does not approve the derived assertion.

23.12.11.10.1. Feature Overview

Cross-facility correlation connects an emergency presentation with relevant recent history and allergy information. It supports situational understanding across institutional boundaries without deciding the clinical significance of the combined evidence or assigning emergency treatment.

23.12.11.10.2. Definition / governed semantic detail

Correlate incoming emergency presentations with recent regional admissions, prior discharge summaries, and known allergy profiles.

23.12.11.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Provide Emergency LHR Summary (p. 558)	Established
From: associated feature	Service: Emergency Fast-Path LHR Query (p. 559)	Established

23.12.11.11. Function: Bind ED Triage Assessment

Property	Value	Meaning
Reference Id	`HCM-FN-bind-ed-triage-assessment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FN-BIND-ED-TRIAGE-ASSESSMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Care Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FN-BIND-ED-TRIAGE-ASSESSMENT`	Direct to this identity; never reusable.
Evidence	sd:95 (p. 1450)	Source standing does not approve the derived assertion.

23.12.11.11.1. Definition / governed semantic detail

Captures ATS (Australasian Triage Scale) category, presenting complaint, and ambulance paramedic handover data.

23.12.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	Established

23.12.11.12. Service: ED Triage Stream Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-ed-triage-stream-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.SV-ED-TRIAGE-STREAM-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Care Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.SV-ED-TRIAGE-STREAM-INGRESS`	Direct to this identity; never reusable.
Evidence	sd:96 (p. 1450)	Source standing does not approve the derived assertion.

23.12.11.12.1. Definition / governed semantic detail

Ingests emergency triage presentations.

23.12.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	Established

23.12.11.13. Function: Provide Emergency LHR Summary

Property	Value	Meaning
Reference Id	`HCM-FN-provide-emergency-lhr-summary`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FN-PROVIDE-EMERGENCY-LHR-SUMMARY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Care Progression	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FN-PROVIDE-EMERGENCY-LHR-SUMMARY`	Direct to this identity; never reusable.
Evidence	sd:98 (p. 1450)	Source standing does not approve the derived assertion.

23.12.11.13.1. Definition / governed semantic detail

Delivers rapid, override-capable access to Advance Care Directives, allergy records, and recent hospital admissions.

23.12.11.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Emergency Care Cross-Facility Correlation (p. 556)	Established

23.12.11.14. Service: Emergency Fast-Path LHR Query

Property	Value	Meaning
Reference Id	`HCM-SV-emergency-fast-path-lhr-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.SV-EMERGENCY-FAST-PATH-LHR-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations → BEC CT3: Emergency Care Progression	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.SV-EMERGENCY-FAST-PATH-LHR-QUERY`	Direct to this identity; never reusable.
Evidence	sd:99 (p. 1450)	Source standing does not approve the derived assertion.

23.12.11.14.1. Definition / governed semantic detail

Provides low-latency emergency record summaries.

23.12.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Emergency Care Cross-Facility Correlation (p. 556)	Established

23.12.12. Function: Coordinate ED Departmental Capacity

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-ed-departmental-capacity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FN-COORDINATE-ED-DEPARTMENTAL-CAPACITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FN-COORDINATE-ED-DEPARTMENTAL-CAPACITY`	Direct to this identity; never reusable.
Evidence	hso:124 (p. 1458); hso:128 (p. 1458)	Source standing does not approve the derived assertion.

23.12.12.1. Definition / governed semantic detail

Understands and manages available, committed and utilised ED capacity in relation to care-place availability, demand, patient presence/progression, acuity and operational constraints; retains waiting room occupancy, resuscitation bay availability and time-to-triage metrics.

Capacity boundary: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.

23.12.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: ED Bed & Bay Tracking (p. 551)	Established
To: realises feature	Feature: Emergency Treatment Capacity Management (p. 553)	Proposed

23.12.13. Service: ED Operational Status Query

Property	Value	Meaning
Reference Id	`HCM-SV-ed-operational-status-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.SV-ED-OPERATIONAL-STATUS-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Emergency Department Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.SV-ED-OPERATIONAL-STATUS-QUERY`	Direct to this identity; never reusable.
Evidence	hso:125 (p. 1458); hso:128 (p. 1458)	Source standing does not approve the derived assertion.

23.12.13.1. Definition / governed semantic detail

Discloses facility ED occupancy and surge level.

Capacity boundary: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.

23.13. BEC CT2: Outpatient Operations

Property	Value	Meaning
Reference Id	`HCM-BEC-outpatient-operations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:452 (p. 1375); enabling:501 (p. 1376); hso:134 (p. 1459)	Source standing does not approve the derived assertion.

23.13.1. Capability Overview

Outpatient Operations integrates arrival events and follow-up requirements generated as clinics progress. It connects a person's operational presence with the relevant service context and makes post-clinic booking or diagnostic needs available for subsequent activity. The responsibility retains outpatient-specific capacity semantics without requiring a separate arrival Function where approved reusable composition suffices.

23.13.2. Definition — Reviewed candidate

Outpatient Operations integrates arrival events and follow-up requirements generated as clinics progress. It connects a person's operational presence with the relevant service context and makes post-clinic booking or diagnostic needs available for subsequent activity. The responsibility retains outpatient-specific capacity semantics without requiring a separate arrival Function where approved reusable composition suffices.

23.13.3. Boundary — retained v0.1 source explanation

An arrival does not establish consultation completion, and a captured follow-up requirement does not perform the clinical activity.

23.13.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Arrival and clinic-outcome information materially connect outpatient progression to care coordination.

23.13.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.13.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.13.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.13.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Outpatient Capacity (p. 567)	function	Canonical / Established
Service: Ambulatory Session Status Query (p. 568)	service	Canonical / Established
Function: Capture Post-Clinic Follow-up Requirements (p. 569)	function	Canonical / Established
BEC CT3: Outpatient Flow Coordination (p. 563)	capability CT3	Proposed / Proposed

23.13.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.13.10. BEC CT3: Outpatient Flow Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-outpatient-flow-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.13.10.1. Capability Overview

Coordinate outpatient arrival, clinic flow, follow-up requirements and context-specific outpatient capacity information. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.13.10.2. Definition — Reviewed candidate

Coordinate outpatient arrival, clinic flow, follow-up requirements and context-specific outpatient capacity information.

23.13.10.3. Intent

Coordinate outpatient arrival, clinic flow, follow-up requirements and context-specific outpatient capacity information.

23.13.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.13.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.13.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-07398864` (line 7484). Working locators are not identity aliases.

23.13.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Outpatient Arrival Registration (p. 564)	feature FT	Canonical / Proposed
Feature: Post-Clinic Order Coordination (p. 565)	feature FT	Canonical / Proposed
Feature: Outpatient Appointment Capacity Management (p. 566)	feature FT	Proposed / Proposed

23.13.10.8. Feature: Outpatient Arrival Registration

Property	Value	Meaning
Reference Id	`FEAT-HSO-09`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-HSO-09`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations → BEC CT3: Outpatient Flow Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FEAT-HSO-09`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Outpatient Operations	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:503 (p. 1377)	Source standing does not approve the derived assertion.

23.13.10.8.1. Feature Overview

Outpatient arrival registration connects a check-in event with the treating context and required notification. Approved reusable composition supplies this integration responsibility; it does not require a specialised arrival Function or prove that a consultation has begun or finished.

23.13.10.8.2. Definition / governed semantic detail

Ingest self-service check-in and clinic arrival events, notifying treating clinicians.

23.13.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.13.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Outpatient Capacity (p. 567)	Established

23.13.10.9. Feature: Post-Clinic Order Coordination

Property	Value	Meaning
Reference Id	`FEAT-HSO-10`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-HSO-10`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations → BEC CT3: Outpatient Flow Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FEAT-HSO-10`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Outpatient Operations	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:504 (p. 1377)	Source standing does not approve the derived assertion.

23.13.10.9.1. Feature Overview

Post-clinic coordination captures the follow-up bookings and diagnostic requirements established at clinic completion. It connects those requirements with subsequent operational activity while preserving that capture is not the clinical decision, a completed booking or diagnostic fulfilment.

23.13.10.9.2. Definition / governed semantic detail

Capture follow-up booking and diagnostic order requirements generated at clinic completion.

23.13.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.13.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Capture Post-Clinic Follow-up Requirements (p. 569)	Established

23.13.10.10. Feature: Outpatient Appointment Capacity Management

Property	Value	Meaning
Reference Id	`HCM-FT-outpatient-appointment-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-STEP2-OUTPATIENT-APPOINTMENT-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations → BEC CT3: Outpatient Flow Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FEAT-STEP2-OUTPATIENT-APPOINTMENT-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

23.13.10.10.1. Feature Overview

The projection distinguishes scheduled slots from currently usable care capacity without taking source booking authority.

23.13.10.10.2. Definition — Proposed

Represent usable outpatient session capacity from booking, staffing and local flow information.

23.13.10.10.3. Boundary

Local responsibility retains its capacity meaning; Service Capacity Management consumes it without acquiring ownership or imposing one common capacity model.

23.13.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.13.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

23.13.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Outpatient Capacity (p. 567)	Proposed

23.13.11. Function: Manage Outpatient Capacity

Property	Value	Meaning
Reference Id	`HCM-FN-manage-outpatient-capacity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.FN-MANAGE-OUTPATIENT-CAPACITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FN-MANAGE-OUTPATIENT-CAPACITY`	Direct to this identity; never reusable.
Name history	Coordinate Outpatient Session Utilisation	Names do not establish another identity.
Evidence	hso:137 (p. 1459); hso:138 (p. 1459); hso:141 (p. 1459); hso:142 (p. 1459)	Source standing does not approve the derived assertion.

23.13.11.1. Definition / governed semantic detail

Understands and manages available, committed and utilised outpatient capacity through clinic/session, practitioner and room availability, appointment demand, patient arrival/waiting and

operational progression. Retains specialty room allocation and clinician arrival tracking.

Exposure qualification: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description. That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.

Capacity boundary: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.

23.13.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Outpatient Arrival Registration (p. 564)	Established
To: realises feature	Feature: Outpatient Appointment Capacity Management (p. 566)	Proposed

23.13.12. Service: Ambulatory Session Status Query

Property	Value	Meaning
Reference Id	`HCM-SV-ambulatory-session-status-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.SV-AMBULATORY-SESSION-STATUS-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.SV-AMBULATORY-SESSION-STATUS-QUERY`	Direct to this identity; never reusable.
Evidence	hso:140 (p. 1459); hso:141 (p. 1459); hso:142 (p. 1459)	Source standing does not approve the derived assertion.

23.13.12.1. Definition / governed semantic detail

Exposes clinic room occupancy.

Exposure qualification: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description. That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.

Capacity boundary: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.

23.13.13. Function: Capture Post-Clinic Follow-up Requirements

Property	Value	Meaning
Reference Id	`HCM-FN-capture-post-clinic-follow-up-requirements`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.FN-CAPTURE-POST-CLINIC-FOLLOW-UP-REQUIREMENTS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Outpatient Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FN-CAPTURE-POST-CLINIC-FOLLOW-UP-REQUIREMENTS`	Direct to this identity; never reusable.
Evidence	hso:144 (p. 1459)	Source standing does not approve the derived assertion.

23.13.13.1. Definition / governed semantic detail

Captures follow-up booking and diagnostic-order requirements generated at clinic completion.

Derivation: `FEAT-HSO-10` directly establishes this capture responsibility. The Clinic queue Process's consultation-concluded checkpoint describes orders placed, not Outpatient Operations' capture of follow-up requirements. Capture does not authorise clinical orders, own authoritative bookings or establish a new Service dependency.

23.13.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Post-Clinic Order Coordination (p. 565)	Established

23.14. BEC CT2: Bed & Care-Place Management

Property	Value	Meaning
Reference Id	`HCM-BEC-bed-care-place-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:453 (p. 1375); enabling:506 (p. 1377); hso:151 (p. 1459)	Source standing does not approve the derived assertion.

23.14.1. Capability Overview

Bed & Care-Place Management maintains operational place state and the turnover progression that makes a place usable again. Vacancy, reservation, cleaning and readiness must remain distinguishable so that an empty bed is not automatically treated as available for allocation. Notifications expose meaningful readiness changes to the responsible operational participants.

23.14.2. Definition — Reviewed candidate

Bed & Care-Place Management maintains operational place state and the turnover progression that makes a place usable again. Vacancy, reservation, cleaning and readiness must remain distinguishable so that an empty bed is not automatically treated as available for allocation. Notifications expose meaningful readiness changes to the responsible operational participants.

23.14.3. Intent — retained v0.1 source explanation

Support credible care-place decisions using governed state rather than physical presence alone.

23.14.4. Boundary — retained v0.1 source explanation

A ready bed does not establish service-level capacity or collective readiness, and place state does not determine clinical suitability or care responsibility.

23.14.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Governed place-state progression and operational notification are substantive assigned platform responsibilities.

23.14.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.14.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.14.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.14.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Maintain Operational Bed State (p. 576)	function	Canonical / Established
Service: Bed Availability Query (p. 577)	service	Canonical / Established
Function: Manage Bed Turnover Progression (p. 578)	function	Canonical / Established
Service: Bed Turnover Lifecycle Service (p. 579)	service	Canonical / Established
Process: Bed Turnover Process (p. 580)	process	Canonical / Established
BEC CT3: Bed & Care-Place State Administration (p. 571)	capability CT3	Proposed / Proposed
BEC CT3: Bed & Care-Place Turnover Coordination (p. 573)	capability CT3	Proposed / Proposed

23.14.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Scheduling & Capacity Management (p. 31)	Proposed
To: enterprise enabled by	Enterprise CT3: Entity-Relative Operational Progression (p. 861)	Proposed

23.14.11. BEC CT3: Bed & Care-Place State Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-bed-care-place-state-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-STATE-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.14.11.1. Capability Overview

Administer the current operational occupancy, reservation, blocking and readiness states of beds and other care-places. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.14.11.2. Definition — Reviewed candidate

Administer the current operational occupancy, reservation, blocking and readiness states of beds and other care-places.

23.14.11.3. Intent

Administer the current operational occupancy, reservation, blocking and readiness states of beds and other care-places.

23.14.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.14.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.14.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-9F88F277` (line 7580). Working locators are not identity aliases.

23.14.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Bed Availability Tracking (p. 572)	feature FT	Canonical / Proposed

23.14.11.8. Feature: Bed Availability Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-11`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-STATE-ADMINISTRATION.FEAT-HSO-11`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management → BEC CT3: Bed & Care-Place State Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FEAT-HSO-11`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Bed & Care-Place Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:508 (p. 1377)	Source standing does not approve the derived assertion.

23.14.11.8.1. Feature Overview

Availability tracking preserves the operational distinctions among vacancy, occupancy, reservation, blockage and cleaning conditions. These states support meaningful care-place coordination without equating an unoccupied bed with a ready place or usable service capacity.

23.14.11.8.2. Definition / governed semantic detail

Maintain operational status of all registered beds and care-places (`VACANT`, `OCCUPIED`, `BLOCKED`, `RESERVED`, `DIRTY`, `CLEANING`, `CLEAN`).

23.14.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.14.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Operational Bed State (p. 576)	Established
From: associated feature	Service: Bed Availability Query (p. 577)	Established

23.14.12. BEC CT3: Bed & Care-Place Turnover Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-bed-care-place-turnover-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.14.12.1. Capability Overview

Coordinate the operational transition of a released care-place through cleaning, preparation and readiness notifications. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.14.12.2. Definition — Reviewed candidate

Coordinate the operational transition of a released care-place through cleaning, preparation and readiness notifications.

23.14.12.3. Intent

Coordinate the operational transition of a released care-place through cleaning, preparation and readiness notifications.

23.14.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.14.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.14.12.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Bed State Progression (p. 575)	feature FT	Canonical / Proposed
Feature: Bed Readiness Notification (p. 576)	feature FT	Canonical / Proposed

23.14.12.7. Feature: Bed State Progression

Property	Value	Meaning
Reference Id	`FEAT-HSO-12`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION.FEAT-HSO-12`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management → BEC CT3: Bed & Care-Place Turnover Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FEAT-HSO-12`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Bed & Care-Place Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:509 (p. 1377)	Source standing does not approve the derived assertion.

23.14.12.7.1. Feature Overview

Bed-state progression applies the consequences of relevant clinical and environmental events to the managed place state. The resulting change is an operational fact, such as a need for cleaning after departure, rather than a transfer of clinical responsibility.

23.14.12.7.2. Definition / governed semantic detail

Progress bed states in response to clinical and environmental events (e.g., patient discharge automatically transitions bed to `DIRTY`).

23.14.12.7.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.14.12.7.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Bed Turnover Progression (p. 578)	Established
From: associated feature	Service: Bed Turnover Lifecycle Service (p. 579)	Established

23.14.12.8. Feature: Bed Readiness Notification

Property	Value	Meaning
Reference Id	`FEAT-HSO-13`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION.FEAT-HSO-13`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management → BEC CT3: Bed & Care-Place Turnover Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FEAT-HSO-13`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Bed & Care-Place Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:510 (p. 1377)	Source standing does not approve the derived assertion.

23.14.12.8.1. Feature Overview

Readiness notification exposes the established readiness of a care place to the responsible allocation participants. It makes a relevant transition actionable without deciding the clinical suitability of the next allocation or establishing whole-service readiness.

23.14.12.8.2. Definition / governed semantic detail

Notify bed allocation managers and clinical units when a cleaned bed achieves `READY` status.

23.14.12.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.14.12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Bed Turnover Progression (p. 578)	Established

23.14.13. Function: Maintain Operational Bed State

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-operational-bed-state`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MAINTAIN-OPERATIONAL-BED-STATE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MAINTAIN-OPERATIONAL-BED-STATE`	Direct to this identity; never reusable.
Evidence	hso:157 (p. 1459)	Source standing does not approve the derived assertion.

23.14.13.1. Definition / governed semantic detail

Tracks real-time bed status and environmental locks.

23.14.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Bed Availability Tracking (p. 572)	Established

23.14.14. Service: Bed Availability Query

Property	Value	Meaning
Reference Id	`HCM-SV-bed-availability-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-AVAILABILITY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-AVAILABILITY-QUERY`	Direct to this identity; never reusable.
Evidence	hso:158 (p. 1459)	Source standing does not approve the derived assertion.

23.14.14.1. Definition / governed semantic detail

Discloses real-time bed availability across hospital directorates.

23.14.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Bed Availability Tracking (p. 572)	Established

23.14.15. Function: Manage Bed Turnover Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-bed-turnover-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MANAGE-BED-TURNOVER-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MANAGE-BED-TURNOVER-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:160 (p. 1459)	Source standing does not approve the derived assertion.

23.14.15.1. Definition / governed semantic detail

Coordinates cleaning, disinfection, and preparation: *Vacated → Cleaning Requested → Cleaning In-Progress → Inspected/Ready → Occupied*.

Additional Feature responsibility: `FEAT-HSO-13 — Bed Readiness Notification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.

23.14.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Bed State Progression (p. 575)	Established
To: realises feature	Feature: Bed Readiness Notification (p. 576)	Established
To: governed process	Process: Bed Turnover Process (p. 580)	Established

23.14.16. Service: Bed Turnover Lifecycle Service

Property	Value	Meaning
Reference Id	`HCM-SV-bed-turnover-lifecycle-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-TURNOVER-LIFECYCLE-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-TURNOVER-LIFECYCLE-SERVICE`	Direct to this identity; never reusable.
Evidence	hso:161 (p. 1459)	Source standing does not approve the derived assertion.

23.14.16.1. Definition / governed semantic detail

Coordinates turnover requests with environmental services.

Additional Feature responsibility: `FEAT-HSO-13 — Bed Readiness Notification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.

23.14.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Bed State Progression (p. 575)	Established

23.14.17. Process: Bed Turnover Process

Property	Value	Meaning
Reference Id	`HCM-PR-bed-turnover-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.PR-BED-TURNOVER-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Bed & Care-Place Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.PR-BED-TURNOVER-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:203 (p. 1419); processes:204 (p. 1419)	Source standing does not approve the derived assertion.

23.14.17.1. Process Overview

Bed turnover connects the release of a care place with the operational work needed to return it to use. Its progression preserves the difference between vacancy, cleaning and readiness. A place can be empty while work remains outstanding, and a completed turnover does not by itself prove that the service has capacity for the next patient.

23.14.17.2. Definition / governed semantic detail

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Process Purpose**: Governs the rapid physical turnover, cleaning, sanitisation, and re-allocation of inpatient and emergency care-places.
- **State Progression Lifecycle**: Bed Vacated → Cleaning Dispatched → Sanitisation In-Progress → Terminal Cleaning Verified → Bed Ready / Available → Bed Allocated / Occupied
- **Key State Dispositions**:
  - `BedVacated`: Prior patient discharged or transferred.
  - `CleaningDispatched`: Cleaning job allocated to environmental services team.
  - `SanitisationIn-Progress`: Ward cleaning or isolation decontamination underway.
  - `TerminalCleaningVerified`: Ward coordinator or supervisor verifies care-place readiness.
  - `BedReady/Available`: Bed marked available for incoming patient allocation.
  - `BedAllocated/Occupied`: Incoming patient allocated and physically placed in bed.

23.14.17.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Bed Turnover Progression (p. 578)	Established

23.15. BEC CT2: Service Capacity Management

Property	Value	Meaning
Reference Id	`HCM-BEC-service-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:455 (p. 1375); enabling:516 (p. 1377); hso:179 (p. 1460)	Source standing does not approve the derived assertion.

23.15.1. Capability Overview

Service Capacity Management provides a higher-order view by aggregating capacity information across services and facilities. It can consider occupancy, waits and diversion while preserving the service-specific meaning of the underlying capacity. Its purpose is not to erase differences between clinic, ward, theatre, emergency, outpatient and mobile-service operation.

23.15.2. Definition — Reviewed candidate

Service Capacity Management provides a higher-order view by aggregating capacity information across services and facilities. It can consider occupancy, waits and diversion while preserving the service-specific meaning of the underlying capacity. Its purpose is not to erase differences between clinic, ward, theatre, emergency, outpatient and mobile-service operation.

23.15.3. Boundary — retained v0.1 source explanation

Consuming contextual capacity does not acquire the originating capacity-management responsibility or infer capacity solely from resource counts.

23.15.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Cross-service operational visibility materially supports coordination while source service contexts retain their authority.

23.15.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.15.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.15.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.15.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Aggregate Service Capacity Telemetry (p. 585)	function	Canonical / Established
Service: Regional Health System Capacity Query (p. 585)	service	Canonical / Established
BEC CT3: Service Demand & Capacity Coordination (p. 582)	capability CT3	Proposed / Proposed

23.15.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Scheduling & Capacity Management (p. 31)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.15.10. BEC CT3: Service Demand & Capacity Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-service-demand-capacity-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.CT3-BEC-SERVICE-DEMAND-CAPACITY-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Service Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.15.10.1. Capability Overview

Coordinate demand and capacity information across services while retaining the originating responsibilities of context-specific capacity providers. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.15.10.2. Definition — Reviewed candidate

Coordinate demand and capacity information across services while retaining the originating responsibilities of context-specific capacity providers.

23.15.10.3. Intent

Coordinate demand and capacity information across services while retaining the originating responsibilities of context-specific capacity providers.

23.15.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.15.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.15.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-D90FAB4C` (line 7779). Working locators are not identity aliases.

23.15.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Operational Capacity Metric Aggregation (p. 584)	feature FT	Canonical / Proposed

23.15.10.8. Feature: Operational Capacity Metric Aggregation

Property	Value	Meaning
Reference Id	FEAT-HSO-15	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.CT3-BEC-SERVICE-DEMAND-CAPACITY-COORDINATION.FEAT-HSO-15	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Service Capacity Management → BEC CT3: Service Demand & Capacity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT.FEAT-HSO-15	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Service Capacity Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:518 (p. 1377)	Source standing does not approve the derived assertion.

23.15.10.8.1. Feature Overview

Capacity aggregation combines occupancy, waiting and diversion context across facilities. The higher-order view must retain the originating service-specific capacity meaning; a shared metric does not transfer clinic, ward or theatre capacity responsibility to the aggregator.

23.15.10.8.2. Definition / governed semantic detail

Aggregate operational capacity telemetry (occupancy rates, wait times, diversion status) across facilities.

23.15.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.15.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Aggregate Service Capacity Telemetry (p. 585)	Established
From: associated feature	Service: Regional Health System Capacity Query (p. 585)	Established

23.15.11. Function: Aggregate Service Capacity Telemetry

Property	Value	Meaning
Reference Id	`HCM-FN-aggregate-service-capacity-telemetry`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.FN-AGGREGATE-SERVICE-CAPACITY-TELEMETRY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Service Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT.FN-AGGREGATE-SERVICE-CAPACITY-TELEMETRY`	Direct to this identity; never reusable.
Evidence	hso:182 (p. 1460); hso:184 (p. 1460)	Source standing does not approve the derived assertion.

23.15.11.1. Definition / governed semantic detail

Consumes and coordinates context-specific service-capacity information for broader situational awareness, alongside bed occupancy, ICU surge capacity and ventilator availability across regional hospitals.

23.15.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Operational Capacity Metric Aggregation (p. 584)	Established

23.15.12. Service: Regional Health System Capacity Query

Property	Value	Meaning
Reference Id	`HCM-SV-regional-health-system-capacity-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.SV-REGIONAL-HEALTH-SYSTEM-CAPACITY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Service Capacity Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT.SV-REGIONAL-HEALTH-SYSTEM-CAPACITY-QUERY`	Direct to this identity; never reusable.
Evidence	hso:183 (p. 1460); hso:184 (p. 1460)	Source standing does not approve the derived assertion.

23.15.12.1. Definition / governed semantic detail

Provides executive operational situational awareness.

23.15.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Operational Capacity Metric Aggregation (p. 584)	Established

23.16. BEC CT2: Service Establishment Support

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-service-establishment-support`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-ESTABLISHMENT-SUPPORT`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.16.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

23.16.2. Capability Overview

Support the information needed to establish a health service in its intended operating context. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.16.3. Definition — Reviewed candidate

Support the information needed to establish a health service in its intended operating context.

23.16.4. Intent

Support the information needed to establish a health service in its intended operating context.

23.16.5. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.16.6. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.17. BEC CT2: Clinical Workforce Capability Monitoring

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-workforce-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-WORKFORCE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-CLINICAL-WORKFORCE-MANAGEMENT`	Direct to this identity; never reusable.

Property	Value	Meaning
Established name	Clinical Workforce Management	Current display rename is Proposed; meaning unchanged.
Name history	Clinical Workforce Management	Names do not establish another identity.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:456 (p. 1375)	Source standing does not approve the derived assertion.

23.17.1. Capability Overview

Monitor supplied information about the clinical capability available from the workforce without acquiring employment or clinical allocation authority.

23.17.2. Definition — Reviewed candidate

Monitor supplied information about the clinical capability available from the workforce without acquiring employment or clinical allocation authority.

23.17.3. Boundary — retained v0.1 source explanation

No new workforce system responsibility or complete CT ancestry is inferred from this contextual entry.

23.17.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Adjacent standing reflects contextual participation rather than management of the workforce itself.

23.17.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.17.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.17.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.18. BEC CT2: Clinical Rostering

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-rostering`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-ROSTERING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-CLINICAL-ROSTERING`	Direct to this identity; never reusable.
Harmonia relevance	Adjacent	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:457 (p. 1375)	Source standing does not approve the derived assertion.

23.18.1. Capability Overview

Clinical Rostering provides the Adjacent context of planned workforce availability. It is distinct from consuming roster projections for operational views or resolving who is currently on call. The present architecture retains the context without establishing a Harmonia Feature decomposition for authoring or managing the authoritative roster.

23.18.2. Definition — Reviewed candidate

Clinical Rostering provides the Adjacent context of planned workforce availability. It is distinct from consuming roster projections for operational views or resolving who is currently on call. The present architecture retains the context without establishing a Harmonia Feature decomposition for authoring or managing the authoritative roster.

23.18.3. Boundary — retained v0.1 source explanation

A projected roster does not confer qualification, privilege, deployment or ownership of the originating schedule.

23.18.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Roster information can inform operations while the formal Adjacent classification preserves its external management.

23.18.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.18.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.18.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.18.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Clinical Roster Capture & Communication (p. 590)	capability CT3	Proposed / Proposed

23.18.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Workforce Management (p. 177)	Proposed

23.18.10. BEC CT3: Clinical Roster Capture & Communication

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-roster-capture-communication`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-ROSTERING.CT3-BEC-CLINICAL-ROSTER-CAPTURE-COMMUNICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Rostering	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.18.10.1. Capability Overview

Capture and communicate supplied clinical roster information, its effective periods and subsequent coverage changes. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.18.10.2. Definition — Reviewed candidate

Capture and communicate supplied clinical roster information, its effective periods and subsequent coverage changes.

23.18.10.3. Intent

Capture and communicate supplied clinical roster information, its effective periods and subsequent coverage changes.

23.18.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.18.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.19. BEC CT2: Mobile Staff Management

Property	Value	Meaning
Reference Id	`HCM-BEC-mobile-staff-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:458 (p. 1375); enabling:520 (p. 1377); hso:190 (p. 1460)	Source standing does not approve the derived assertion.

23.19.1. Capability Overview

Mobile Staff Management integrates with external business mobile-work mechanisms and receives visit progression and safety milestones. Its established dispatch Feature exchanges assignment, dispatch and status information with the relevant worker context. This is an information integration boundary around mobile activity, rather than a platform claim to decide the clinician's work.

23.19.2. Definition — Reviewed candidate

Mobile Staff Management integrates with external business mobile-work mechanisms and receives visit progression and safety milestones. Its established dispatch Feature exchanges assignment, dispatch and status information with the relevant worker context. This is an information integration boundary around mobile activity, rather than a platform claim to decide the clinician's work.

23.19.3. Boundary — retained v0.1 source explanation

Harmonia does not determine clinical allocation, handover or EMR worklists. External work/task language does not redefine the internal Synthetic Task.

23.19.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Mobile assignment and progression exchanges materially connect roving activity to the wider care environment within that bounded integration role.

23.19.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.19.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.19.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.19.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Mobile Service Capacity (p. 597)	function	Canonical / Established
Function: Track Operational Staff Presence (p. 598)	function	Canonical / Established
Service: Staff Presence Telemetry Query (p. 599)	service	Canonical / Established
Function: Receive Mobile Visit Progression (p. 600)	function	Canonical / Established
Function: Integrate Mobile Work Management (p. 601)	function	Canonical / Established
BEC CT3: Mobile Worker Dispatch & Safety Coordination (p. 593)	capability CT3	Proposed / Proposed

23.19.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

23.19.10. BEC CT3: Mobile Worker Dispatch & Safety Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-mobile-worker-dispatch-safety-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.19.10.1. Capability Overview

Exchange externally established mobile work assignments, visit progression and safety check-in information with qualified worker context. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.19.10.2. Definition — Reviewed candidate

Exchange externally established mobile work assignments, visit progression and safety check-in information with qualified worker context.

23.19.10.3. Intent

Exchange externally established mobile work assignments, visit progression and safety check-in information with qualified worker context.

23.19.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.19.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.19.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-EC43C4CB` (line 7875). Working locators are not identity aliases.

23.19.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Mobile Worker Task Dispatch (p. 594)	feature FT	Canonical / Proposed
Feature: Mobile Visit Status Tracking (p. 595)	feature FT	Canonical / Proposed
Feature: Mobile Service Capacity Management (p. 596)	feature FT	Proposed / Proposed

23.19.10.8. Feature: Mobile Worker Task Dispatch

Property	Value	Meaning
Reference Id	`FEAT-HSO-16`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-HSO-16`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management → BEC CT3: Mobile Worker Dispatch & Safety Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FEAT-HSO-16`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Mobile Staff Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:522 (p. 1377); enabling:523 (p. 1377)	Source standing does not approve the derived assertion.

23.19.10.8.1. Feature Overview

Mobile dispatch exchanges externally established assignment, dispatch and progression information with business mobile-work mechanisms. The retained name does not grant Harmonia authority to allocate clinical work, manage handover or own EMR worklists, and external task terminology does not redefine Synthetic Task.

23.19.10.8.2. Definition / governed semantic detail

Dispatch community care tasks and urgent visits to roving clinicians' mobile devices.

23.19.10.8.3. Boundary

The proposed replacement name remains unconfirmed. This entry preserves the existing identity and the human-adjudicated integration boundary.

23.19.10.8.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.19.10.8.5. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Integrate Mobile Work Management (p. 601)	Established
From: realises feature	Function: Track Operational Staff Presence (p. 598)	Unresolved association
From: associated feature	Service: Staff Presence Telemetry Query (p. 599)	Unresolved association

23.19.10.9. Feature: Mobile Visit Status Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-17`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-HSO-17`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management → BEC CT3: Mobile Worker Dispatch & Safety Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FEAT-HSO-17`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Mobile Staff Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:524 (p. 1377)	Source standing does not approve the derived assertion.

23.19.10.9.1. Feature Overview

Mobile visit tracking receives arrival, progression and safety checkpoints from roving workers. It makes reported activity visible in the wider operational context without originating the clinical visit, deciding its clinical content or assuming that a checkpoint proves the intended clinical outcome.

23.19.10.9.2. Definition / governed semantic detail

Ingest arrival, visit progression, and safety check-in milestones from roving staff.

23.19.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.19.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Mobile Visit Progression (p. 600)	Established

23.19.10.10. Feature: Mobile Service Capacity Management

Property	Value	Meaning
Reference Id	`HCM-FT-mobile-service-capacity-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-STEP2-MOBILE-SERVICE-CAPACITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management → BEC CT3: Mobile Worker Dispatch & Safety Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FEAT-STEP2-MOBILE-SERVICE-CAPACITY-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

23.19.10.10.1. Feature Overview

Mobile availability cannot be interpreted as a simple on-duty count because distance, assignments and suitable skills affect deliverability.

23.19.10.10.2. Definition — Proposed

Represent mobile service capacity with travel, coverage, work commitment and availability context.

23.19.10.10.3. Boundary

Local responsibility retains its capacity meaning; Service Capacity Management consumes it without acquiring ownership or imposing one common capacity model.

23.19.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.19.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

23.19.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Mobile Service Capacity (p. 597)	Proposed

23.19.11. Function: Manage Mobile Service Capacity

Property	Value	Meaning
Reference Id	`HCM-FN-manage-mobile-service-capacity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-MANAGE-MOBILE-SERVICE-CAPACITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-MANAGE-MOBILE-SERVICE-CAPACITY`	Direct to this identity; never reusable.
Evidence	hso:193 (p. 1460); hso:194 (p. 1460)	Source standing does not approve the derived assertion.

23.19.11.1. Definition / governed semantic detail

Understands and manages available, committed and utilised mobile-service capacity in relation to workforce availability, geography, time, workload and applicable service demand.

Responsibility boundary: Staff presence and externally established assignments/visit progression contribute to capacity; they do not alone express it. Capacity coordination does not determine clinical work, own the source workforce roster or allocate underlying clinical duties.

23.19.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Mobile Service Capacity Management (p. 596)	Proposed

23.19.12. Function: Track Operational Staff Presence

Property	Value	Meaning
Reference Id	`HCM-FN-track-operational-staff-presence`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-TRACK-OPERATIONAL-STAFF-PRESENCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-TRACK-OPERATIONAL-STAFF-PRESENCE`	Direct to this identity; never reusable.
Evidence	hso:197 (p. 1460); hso:196 (p. 1460)	Source standing does not approve the derived assertion.

23.19.12.1. Definition / governed semantic detail

Tracks active on-duty presence and physical zone assignment for porters, phlebotomists, and roving nurses.

Feature association not established: `FEAT-HSO-16 — Mobile Worker Task Dispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.

23.19.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Mobile Worker Task Dispatch (p. 594)	Unresolved association

23.19.13. Service: Staff Presence Telemetry Query

Property	Value	Meaning
Reference Id	`HCM-SV-staff-presence-telemetry-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.SV-STAFF-PRESENCE-TELEMETRY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.SV-STAFF-PRESENCE-TELEMETRY-QUERY`	Direct to this identity; never reusable.
Evidence	hso:198 (p. 1460); hso:196 (p. 1460)	Source standing does not approve the derived assertion.

23.19.13.1. Definition / governed semantic detail

Discloses on-duty staff availability by hospital zone.

Feature association not established: `FEAT-HSO-16 — Mobile Worker Task Dispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.

23.19.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Mobile Worker Task Dispatch (p. 594)	Unresolved association

23.19.14. Function: Receive Mobile Visit Progression

Property	Value	Meaning
Reference Id	`HCM-FN-receive-mobile-visit-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-RECEIVE-MOBILE-VISIT-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-RECEIVE-MOBILE-VISIT-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:200 (p. 1460)	Source standing does not approve the derived assertion.

23.19.14.1. Definition / governed semantic detail

Ingests arrival, visit-progression and safety check-in milestones from roving staff.

Derivation: `FEAT-HSO-17` directly requires visit-specific milestone ingestion. It remains distinct from presence, capacity assessment and work-management integration. Received progression can inform mobile capacity without determining clinical work or clinical completion.

23.19.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Mobile Visit Status Tracking (p. 595)	Established

23.19.15. Function: Integrate Mobile Work Management

Property	Value	Meaning
Reference Id	`HCM-FN-integrate-mobile-work-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-INTEGRATE-MOBILE-WORK-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Mobile Staff Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-INTEGRATE-MOBILE-WORK-MANAGEMENT`	Direct to this identity; never reusable.
Evidence	hso:203 (p. 1460); hso:204 (p. 1460); hso:205 (p. 1460)	Source standing does not approve the derived assertion.

23.19.15.1. Definition / governed semantic detail

Exchanges externally established work/assignment, dispatch and operational progression/status information, with applicable worker/context information, between mobile work-management mechanisms and Harmonia operational coordination.

Derivation and boundary: The human adjudication of `FEAT-HSO-16` establishes integration with mobile work/task management. This responsibility is absent from presence observation and visit-specific milestone ingestion. It does not determine, allocate or manage the underlying clinical work, clinical handover or EMR worklists. Work Allocation & Dispatch retains non-clinical operational Work Order responsibility.

23.19.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Mobile Worker Task Dispatch (p. 594)	Established

23.20. BEC CT2: On-Call Management

Property	Value	Meaning
Reference Id	`HCM-BEC-on-call-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:459 (p. 1375); enabling:526 (p. 1377); hso:211 (p. 1461)	Source standing does not approve the derived assertion.

23.20.1. Capability Overview

On-Call Management resolves the active provider for a specialty, service or hospital at the relevant time. It turns changing coverage information into usable routing context so that operational communication can reach the intended capacity. This differs from authoring the roster or deciding who is professionally qualified for the requested activity.

23.20.2. Definition — Reviewed candidate

On-Call Management resolves the active provider for a specialty, service or hospital at the relevant time. It turns changing coverage information into usable routing context so that operational communication can reach the intended capacity. This differs from authoring the roster or deciding who is professionally qualified for the requested activity.

23.20.3. Boundary — retained v0.1 source explanation

Being on call does not by itself establish clinical privilege or assignment of a particular care responsibility.

23.20.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Current provider resolution materially enables appropriate contact and routing across operational boundaries.

23.20.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.20.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.20.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.20.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Resolve Active On-Call Coverage (p. 605)	function	Canonical / Established
Service: On-Call Specialist Lookup (p. 606)	service	Canonical / Established
BEC CT3: On-Call Availability & Contact Resolution (p. 603)	capability CT3	Proposed / Proposed

23.20.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed

23.20.10. BEC CT3: On-Call Availability & Contact Resolution

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-on-call-availability-contact-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.CT3-BEC-ON-CALL-AVAILABILITY-CONTACT-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: On-Call Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.20.10.1. Capability Overview

Resolve currently available on-call providers and their contact paths for the applicable specialty, service and time. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.20.10.2. Definition — Reviewed candidate

Resolve currently available on-call providers and their contact paths for the applicable specialty, service and time.

23.20.10.3. Intent

Resolve currently available on-call providers and their contact paths for the applicable specialty, service and time.

23.20.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.20.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.20.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-4BE71CB3` (line 7984). Working locators are not identity aliases.

23.20.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Active On-Call Provider Resolution (p. 604)	feature FT	Canonical / Proposed

23.20.10.8. Feature: Active On-Call Provider Resolution

Property	Value	Meaning
Reference Id	`FEAT-HSO-18`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.CT3-BEC-ON-CALL-AVAILABILITY-CONTACT-RESOLUTION.FEAT-HSO-18`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: On-Call Management → BEC CT3: On-Call Availability & Contact Resolution	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT.FEAT-HSO-18`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: On-Call Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:528 (p. 1377)	Source standing does not approve the derived assertion.

23.20.10.8.1. Feature Overview

Active on-call resolution identifies the current provider in the required specialty and hospital context. It supports correct routing at the relevant time without authoring the roster or treating current coverage as proof of qualification, privilege or responsibility acceptance.

23.20.10.8.2. Definition / governed semantic detail

Resolve currently active on-call specialists, registrars, and operational managers for a given specialty and hospital at any point in time.

23.20.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.20.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Resolve Active On-Call Coverage (p. 605)	Established
From: associated feature	Service: On-Call Specialist Lookup (p. 606)	Established

23.20.11. Function: Resolve Active On-Call Coverage

Property	Value	Meaning
Reference Id	`HCM-FN-resolve-active-on-call-coverage`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.FN-RESOLVE-ACTIVE-ON-CALL-COVERAGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: On-Call Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT.FN-RESOLVE-ACTIVE-ON-CALL-COVERAGE`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	hso:214 (p. 1461)	Source standing does not approve the derived assertion.

23.20.11.1. Definition / governed semantic detail

Resolves primary, secondary, and tertiary on-call clinical specialists for emergency escalation.

23.20.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Active On-Call Provider Resolution (p. 604)	Established

23.20.12. Service: On-Call Specialist Lookup

Property	Value	Meaning
Reference Id	`HCM-SV-on-call-specialist-lookup`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.SV-ON-CALL-SPECIALIST-LOOKUP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: On-Call Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT.SV-ON-CALL-SPECIALIST-LOOKUP`	Direct to this identity; never reusable.
Evidence	hso:215 (p. 1461)	Source standing does not approve the derived assertion.

23.20.12.1. Definition / governed semantic detail

Discloses active on-call contacts for switchboard and emergency teams.

23.20.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Active On-Call Provider Resolution (p. 604)	Established

23.21. BEC CT2: Work Allocation & Dispatch

Property	Value	Meaning
Reference Id	`HCM-BEC-work-allocation-dispatch`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:460 (p. 1375); enabling:530 (p. 1378); hso:221 (p. 1462)	Source standing does not approve the derived assertion.

23.21.1. Capability Overview

Work Allocation & Dispatch progresses authorised operational work from a recognised requirement through matching, dispatch, acknowledgement and oversight. It distinguishes an instantiated work item from its assignment and from evidence of performance, allowing stalled activity to be identified and escalated. Examples such as cleaning or transport concern real-world support work, not technical message queues.

23.21.2. Definition — Reviewed candidate

Work Allocation & Dispatch progresses authorised operational work from a recognised requirement through matching, dispatch, acknowledgement and oversight. It distinguishes an instantiated work item from its assignment and from evidence of performance, allowing stalled activity to be identified and escalated. Examples such as cleaning or transport concern real-world support work, not technical message queues.

23.21.3. Intent — retained v0.1 source explanation

Make operational work progression accountable from requirement to reported outcome within applicable policy and role constraints.

23.21.4. Boundary — retained v0.1 source explanation

This assignment responsibility does not confer clinical work allocation, qualification or privilege. A technical receipt has only its interaction contract’s meaning.

23.21.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Governed operational work progression is a substantive platform responsibility supporting the Core classification.

23.21.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.21.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.21.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.21.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Dispatch Operational Work Order (p. 614)	function	Canonical / Established
Service: Work Dispatch Service (p. 614)	service	Canonical / Established
Function: Manage Operational Work Progression (p. 615)	function	Canonical / Established
Service: Operational Work Status Query (p. 616)	service	Canonical / Established
Process: Operational Work Progression Process (p. 617)	process	Canonical / Established
BEC CT3: Operational Work Assignment & Dispatch Coordination (p. 608)	capability CT3	Proposed / Proposed

23.21.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Support Logistics Management (p. 41)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed

23.21.11. BEC CT3: Operational Work Assignment & Dispatch Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-operational-work-assignment-dispatch-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.21.11.1. Capability Overview

Coordinate qualified operational work assignment, dispatch, acknowledgement, progression and escalation within assigned authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.21.11.2. Definition — Reviewed candidate

Coordinate qualified operational work assignment, dispatch, acknowledgement, progression and escalation within assigned authority.

23.21.11.3. Intent

Coordinate qualified operational work assignment, dispatch, acknowledgement, progression and escalation within assigned authority.

23.21.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.21.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.21.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-09C6941C` (line 8060). Working locators are not identity aliases.

23.21.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Work Item Instantiation (p. 610)	feature FT	Canonical / Proposed
Feature: Worker Matching & Allocation (p. 611)	feature FT	Canonical / Proposed
Feature: Work Dispatch Delivery (p. 612)	feature FT	Canonical / Proposed
Feature: Work Progress Oversight & Escalation (p. 613)	feature FT	Canonical / Proposed

23.21.11.8. Feature: Work Item Instantiation

Property	Value	Meaning
Reference Id	`FEAT-HSO-19`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-19`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch → BEC CT3: Operational Work Assignment & Dispatch Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-19`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Work Allocation & Dispatch	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:532 (p. 1378)	Source standing does not approve the derived assertion.

23.21.11.8.1. Feature Overview

Work-item instantiation gives an authorised operational requirement an identifiable unit of work. It distinguishes recognising that support activity is needed from assigning it, dispatching it and recording its completion; a clinical trigger does not make the resulting support item clinical work authority.

23.21.11.8.2. Definition / governed semantic detail

Instantiate operational units of work from clinical triggers (e.g., bed cleaning required, specimen transport required).

23.21.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.21.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Dispatch Operational Work Order (p. 614)	Established
From: associated feature	Service: Work Dispatch Service (p. 614)	Established

23.21.11.9. Feature: Worker Matching & Allocation

Property	Value	Meaning
Reference Id	`FEAT-HSO-20`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-20`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch → BEC CT3: Operational Work Assignment & Dispatch Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-20`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Work Allocation & Dispatch	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:533 (p. 1378)	Source standing does not approve the derived assertion.

23.21.11.9.1. Feature Overview

Worker matching relates an operational requirement to eligible roles, credentials, locations and availability under policy. It consumes those constraints rather than changing their authoritative records, and does not turn operational allocation into permission to decide clinical work.

23.21.11.9.2. Definition / governed semantic detail

Match work items against available staff roles, credentials, and locations based on allocation policies.

23.21.11.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.21.11.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Dispatch Operational Work Order (p. 614)	Established

23.21.11.10. Feature: Work Dispatch Delivery

Property	Value	Meaning
Reference Id	FEAT-HSO-21	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-21	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch → BEC CT3: Operational Work Assignment & Dispatch Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-21	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Work Allocation & Dispatch	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:534 (p. 1378)	Source standing does not approve the derived assertion.

23.21.11.10.1. Feature Overview

Dispatch delivery brings assigned operational work to the intended worker or team and retains acknowledgement evidence. Technical receipt and operational acknowledgement must remain distinguishable; neither alone proves that the work was performed or its intended outcome achieved.

23.21.11.10.2. Definition / governed semantic detail

Deliver work orders to target workers, devices, or team queues with technical and operational acknowledgement capture.

23.21.11.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.21.11.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Dispatch Operational Work Order (p. 614)	Established

23.21.11.11. Feature: Work Progress Oversight & Escalation

Property	Value	Meaning
Reference Id	FEAT-HSO-22	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-22	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch → BEC CT3: Operational Work Assignment & Dispatch Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-22	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Work Allocation & Dispatch	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:535 (p. 1378)	Source standing does not approve the derived assertion.

23.21.11.11.1. Feature Overview

Progress oversight identifies stalled or unacknowledged operational work against applicable timing expectations. Escalation serves completion management and does not itself constitute independent assurance or transfer authority over clinical decisions related to the support activity.

23.21.11.11.2. Definition / governed semantic detail

Monitor work progression against time-to-completion thresholds, escalating stalled or unacknowledged tasks.

23.21.11.11.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.21.11.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Operational Work Progression (p. 615)	Established
From: associated feature	Service: Operational Work Status Query (p. 616)	Established

23.21.12. Function: Dispatch Operational Work Order

Property	Value	Meaning
Reference Id	`HCM-FN-dispatch-operational-work-order`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.FN-DISPATCH-OPERATIONAL-WORK-ORDER`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FN-DISPATCH-OPERATIONAL-WORK-ORDER`	Direct to this identity; never reusable.
Evidence	hso:227 (p. 1462)	Source standing does not approve the derived assertion.

23.21.12.1. Definition / governed semantic detail

Allocates work requests to available wardspersons or support teams using applicable allocation policies, staff roles, credentials, location and skill, and delivers allocated work to the target worker, device or team queue.

23.21.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Work Item Instantiation (p. 610)	Established
To: realises feature	Feature: Worker Matching & Allocation (p. 611)	Established
To: realises feature	Feature: Work Dispatch Delivery (p. 612)	Established

23.21.13. Service: Work Dispatch Service

Property	Value	Meaning
Reference Id	`HCM-SV-work-dispatch-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.SV-WORK-DISPATCH-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.SV-WORK-DISPATCH-SERVICE`	Direct to this identity; never reusable.
Evidence	hso:228 (p. 1462)	Source standing does not approve the derived assertion.

23.21.13.1. Definition / governed semantic detail

Delivers allocated work and captures distinct technical and operational acknowledgements; acknowledgement alone does not establish completion.

23.21.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Work Item Instantiation (p. 610)	Established

23.21.14. Function: Manage Operational Work Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-operational-work-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.FN-MANAGE-OPERATIONAL-WORK-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FN-MANAGE-OPERATIONAL-WORK-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:231 (p. 1462)	Source standing does not approve the derived assertion.

23.21.14.1. Definition / governed semantic detail

Tracks execution states: *Queued → Assigned → Accepted → In-Progress → Completed / Aborted*.

23.21.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Work Progress Oversight & Escalation (p. 613)	Established
To: governed process	Process: Operational Work Progression Process (p. 617)	Established

23.21.15. Service: Operational Work Status Query

Property	Value	Meaning
Reference Id	`HCM-SV-operational-work-status-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.SV-OPERATIONAL-WORK-STATUS-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.SV-OPERATIONAL-WORK-STATUS-QUERY`	Direct to this identity; never reusable.
Evidence	hso:232 (p. 1462)	Source standing does not approve the derived assertion.

23.21.15.1. Definition / governed semantic detail

Discloses task progression to requesting wards.

23.21.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Work Progress Oversight & Escalation (p. 613)	Established

23.21.16. Process: Operational Work Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-operational-work-progression-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.PR-OPERATIONAL-WORK-PROGRESSION-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Work Allocation & Dispatch	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.PR-OPERATIONAL-WORK-PROGRESSION-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:218 (p. 1419); processes:219 (p. 1419)	Source standing does not approve the derived assertion.

23.21.16.1. Process Overview

Operational work progression makes the path from a recognised support requirement to assigned and reported work explicit. Acknowledgement and activity progress help distinguish a dispatch from actual performance and expose impediments requiring operational attention. This responsibility remains bounded to authorised support work rather than ownership of the clinician's worklist.

23.21.16.2. Definition / governed semantic detail

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Process Purpose**: Governs the dispatch, assignment, execution, and completion of facility operational jobs (e.g., equipment moves, waste removal, linen supply).
- **State Progression Lifecycle**: Work Requested → Queued in Dispatch → Assigned to Worker → Job Accepted → Work In-Progress → Work Completed / Aborted
- **Key State Dispositions**:
  - `WorkRequested`: Facility support task submitted by clinical unit.
  - `QueuedinDispatch`: Job prioritized in operational queue.
  - `AssignedtoWorker`: Job allocated to specific staff member or team.
  - `JobAccepted`: Worker acknowledges dispatch on mobile device.
  - `WorkIn-Progress`: Task actively being executed.
  - `WorkCompleted/Aborted`: Job marked finished or formally cancelled with reason.

23.21.16.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Operational Work Progression (p. 615)	Established

23.22. BEC CT2: Clinical Training & Education

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-training-education`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-TRAINING-EDUCATION`	Current proposed context where marked Proposed.
Release scope	**Not Addressed in R1.x / R2.x**	Scope annotation; independent of architectural status.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-TRAINING-EDUCATION`	Direct to this identity; never reusable.
Harmonia relevance	Reference	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:461 (p. 1375)	Source standing does not approve the derived assertion.

23.22.1. R1.x / R2.x scope

Not Addressed in R1.x / R2.x. This is a release-scope annotation, not Candidate status or missing architecture.

23.22.2. Capability Overview

Clinical Training & Education remains a Reference context for developing practitioners' knowledge and skills. The catalogue recognises its enterprise significance without assigning training delivery or a Feature decomposition to Harmonia. Its separation from qualification checks prevents a represented certificate or training record from being mistaken for the education activity itself.

23.22.3. Definition — Reviewed candidate

Clinical Training & Education remains a Reference context for developing practitioners' knowledge and skills. The catalogue recognises its enterprise significance without assigning training delivery or a Feature decomposition to Harmonia. Its separation from qualification checks prevents a represented certificate or training record from being mistaken for the education activity itself.

23.22.4. Boundary — retained v0.1 source explanation

No platform training programme, assessment process or clinical teaching authority is established.

23.22.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Reference standing retains enterprise context without a material assigned processing responsibility in this entry.

23.22.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.22.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.22.8. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

23.22.9. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.23. BEC CT2: Clinical Competency Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-competency-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-COMPETENCY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-COMPETENCY-MANAGEMENT`	Direct to this identity; never reusable.
Established name	Clinical Competency Management	Current display rename is Proposed; meaning unchanged.
Name history	Clinical Competency Management	Names do not establish another identity.
Harmonia relevance	Reference	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:462 (p. 1375)	Source standing does not approve the derived assertion.

23.23.1. Capability Overview

Administer information about what clinical capability an organisation can provide and what a practitioner is competent to perform.

23.23.2. Definition — Reviewed candidate

Administer information about what clinical capability an organisation can provide and what a practitioner is competent to perform.

23.23.3. Boundary — retained v0.1 source explanation

No atomic Features or competency-assessment authority are allocated to this context.

23.23.4. Harmonia Architectural Relevance — retained v0.1 source explanation

The formal Reference classification remains separate from the relevance of operational qualification checking elsewhere.

23.23.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.23.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.23.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

23.23.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.24. BEC CT2: Clinical Qualification Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-qualification-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-QUALIFICATION-MANAGEMENT`	Direct to this identity; never reusable.
Established name	Clinical Qualification Management	Current display rename is Proposed; meaning unchanged.
Name history	Clinical Credential Management; Clinical Qualification Management	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:463 (p. 1375); enabling:539 (p. 1378); hso:292 (p. 1464); metamodel:397 (p. 1396)	Source standing does not approve the derived assertion.

23.24.1. Capability Overview

Administer organisation clinical certification/compliance information and provider qualification evidence with its externally established standing.

23.24.2. Definition — Reviewed candidate

Administer organisation clinical certification/compliance information and provider qualification evidence with its externally established standing.

23.24.3. Boundary — retained v0.1 source explanation

The historical name Clinical Credential Management is an alias only for this element. It is not equivalent to Credential Management, and role resolution alone establishes neither clinical nor operational privilege.

23.24.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Qualification context materially constrains legitimate dispatch without making Harmonia the originating professional regulator or training authority.

23.24.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.24.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.24.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

23.24.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.24.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Verify Clinical Qualification for Work Dispatch (p. 627)	function	Canonical / Established
BEC CT3: Organisation Clinical Certification & Compliance Support (p. 622)	capability CT3	Proposed / Proposed
BEC CT3: Provider Clinical Qualification Tracking (p. 623)	capability CT3	Proposed / Proposed

23.24.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Workforce Management (p. 177)	Proposed

23.24.11. BEC CT3: Organisation Clinical Certification & Compliance Support

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-organisation-clinical-certification-compliance-support`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.CT3-BEC-ORGANISATION-CLINICAL-CERTIFICATION-COMPLIANCE-SUPPORT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Qualification Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.24.11.1. Capability Overview

Administer supplied organisation clinical certification and conformance evidence for use in service qualification and participation. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.24.11.2. Definition — Reviewed candidate

Administer supplied organisation clinical certification and conformance evidence for use in service qualification and participation.

23.24.11.3. Intent

Administer supplied organisation clinical certification and conformance evidence for use in service qualification and participation.

23.24.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.24.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.24.12. BEC CT3: Provider Clinical Qualification Tracking

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-provider-clinical-qualification-tracking`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.CT3-BEC-PROVIDER-CLINICAL-QUALIFICATION-TRACKING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Qualification Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.24.12.1. Capability Overview

Track supplied provider clinical qualifications, competency certificates and privilege evidence with effective dates and issuing-authority standing. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.24.12.2. Definition — Reviewed candidate

Track supplied provider clinical qualifications, competency certificates and privilege evidence with effective dates and issuing-authority standing.

23.24.12.3. Intent

Track supplied provider clinical qualifications, competency certificates and privilege evidence with effective dates and issuing-authority standing.

23.24.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.24.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.24.12.6. Consolidated / decomposed predecessors

BEC CT3: Provider Engagement (p. 926)

23.24.12.7. Prior Candidate Derivation

v0.3 reconciliation: `BEC-E3152A69` (line 8213). Working locators are not identity aliases.

23.24.12.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Clinical Privileging Record (p. 625)	feature FT	Canonical / Proposed
Feature: Operational Credential Check (p. 626)	feature FT	Canonical / Proposed

23.24.12.9. Feature: Clinical Privileging Record

Property	Value	Meaning
Reference Id	`FEAT-EM-16`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.CT3-BEC-PROVIDER-CLINICAL-QUALIFICATION-TRACKING.FEAT-EM-16`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Qualification Administration → BEC CT3: Provider Clinical Qualification Tracking	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-ENGAGEMENT.FEAT-EM-16`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Provider Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:156 (p. 1366)	Source standing does not approve the derived assertion.

23.24.12.9.1. Feature Overview

The privileging record makes applicable admitting and practice permissions available as contextual evidence. It represents the health service's established privilege standing rather than granting new authority merely because a record exists in the directory.

23.24.12.9.2. Definition / governed semantic detail

Track active clinical admitting and practice privileges within healthcare facilities.

23.24.12.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.24.12.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Clinical Privileges (p. 261)	Established
From: associated feature	Service: Clinical Privilege Verification (p. 262)	Established

23.24.12.10. Feature: Operational Credential Check

Property	Value	Meaning
Reference Id	FEAT-HSO-23	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.CT3-BEC-PROVIDER-CLINICAL-QUALIFICATION-TRACKING.FEAT-HSO-23	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Qualification Administration → BEC CT3: Provider Clinical Qualification Tracking	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FEAT-HSO-23	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Qualification Administration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:541 (p. 1378)	Source standing does not approve the derived assertion.

23.24.12.10.1. Feature Overview

The qualification checkpoint considers active professional credentials and competency evidence before specialised dispatch. Its historical credential terminology does not make it the operational badge or physical-access responsibility of Credential Management, nor does a qualification check alone establish all required privilege.

23.24.12.10.2. Definition / governed semantic detail

Validate that a practitioner possesses active credentials and required competency certs prior to dispatching specialized work.

23.24.12.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.24.12.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Verify Clinical Qualification for Work Dispatch (p. 627)	Established

23.24.13. Function: Verify Clinical Qualification for Work Dispatch

Property	Value	Meaning
Reference Id	`HCM-FN-verify-clinical-qualification-for-work-dispatch`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FN-VERIFY-CLINICAL-QUALIFICATION-FOR-WORK-DISPATCH`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Qualification Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FN-VERIFY-CLINICAL-QUALIFICATION-FOR-WORK-DISPATCH`	Direct to this identity; never reusable.
Evidence	hso:295 (p. 1464); hso:297 (p. 1464)	Source standing does not approve the derived assertion.

23.24.13.1. Definition / governed semantic detail

Validates that a practitioner possesses active clinical credentials and required competency certificates before dispatch of specialised work.

Derivation: `FEAT-HSO-23` belongs to Strategy Clinical Qualification Management. Provider Administration establishes professional qualification/registration and privilege information, but the dispatch-specific qualification/competency checkpoint was not expressed by that information management alone.

23.24.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Operational Credential Check (p. 626)	Established

23.25. BEC CT2: Credential Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-credential-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CREDENTIAL-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.25.1. Capability Overview

Administer definitions of clinical-service credentials and their required applicability without granting professional privilege or managing physical badges. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.25.2. Definition — Reviewed candidate

Administer definitions of clinical-service credentials and their required applicability without granting professional privilege or managing physical badges.

23.25.3. Intent

Administer definitions of clinical-service credentials and their required applicability without granting professional privilege or managing physical badges.

23.25.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.25.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.25.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Clinical Service Credential Definition Management (p. 629)	capability CT3	Proposed / Proposed

23.25.7. BEC CT3: Clinical Service Credential Definition Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-service-credential-definition-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CREDENTIAL-MANAGEMENT.CT3-BEC-CLINICAL-SERVICE-CREDENTIAL-DEFINITION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Credential Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.25.7.1. Capability Overview

Manage the definitions of credentials required for participation in a clinical service, distinct from physical badges and authentication credentials. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.25.7.2. Definition — Reviewed candidate

Manage the definitions of credentials required for participation in a clinical service, distinct from physical badges and authentication credentials.

23.25.7.3. Intent

Manage the definitions of credentials required for participation in a clinical service, distinct from physical badges and authentication credentials.

23.25.7.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.25.7.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.26. BEC CT2: Clinical Supervision

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-supervision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-SUPERVISION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-SUPERVISION`	Direct to this identity; never reusable.
Harmonia relevance	Reference	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:464 (p. 1375)	Source standing does not approve the derived assertion.

23.26.1. Capability Overview

Enable responsible clinical participants to identify and use the applicable clinical escalation path.

23.26.2. Definition — Reviewed candidate

Enable responsible clinical participants to identify and use the applicable clinical escalation path.

23.26.3. Boundary — retained v0.1 source explanation

Workflow oversight or platform escalation does not confer clinical supervision or responsibility for clinical adequacy.

23.26.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Reference standing is preserved; no direct platform supervision responsibility is established.

23.26.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.26.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.26.7. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

23.26.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.26.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Clinical Escalation Path Management (p. 631)	capability CT3	Proposed / Proposed

23.26.10. BEC CT3: Clinical Escalation Path Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-escalation-path-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-SUPERVISION.CT3-BEC-CLINICAL-ESCALATION-PATH-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Supervision	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.26.10.1. Capability Overview

Manage clinical escalation paths and their applicability, contacts and progression conditions for use by responsible clinical participants. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.26.10.2. Definition — Reviewed candidate

Manage clinical escalation paths and their applicability, contacts and progression conditions for use by responsible clinical participants.

23.26.10.3. Intent

Manage clinical escalation paths and their applicability, contacts and progression conditions for use by responsible clinical participants.

23.26.10.4. Boundary

Harmonia manages the escalation path; responsible clinical participants perform supervision and retain clinical judgement.

23.26.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.27. BEC CT2: Patient Transport

Property	Value	Meaning
Reference Id	`HCM-BEC-patient-transport`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:465 (p. 1375); enabling:543 (p. 1378); hso:250 (p. 1462)	Source standing does not approve the derived assertion.

23.27.1. Capability Overview

Patient Transport coordinates the operational progression of movement requests, including the patient's applicable mobility constraints. Departure, transit and arrival checkpoints make the

physical journey visible to participants. The responsibility supports internal and inter-facility movement without treating arrival as acceptance of clinical responsibility at the destination.

23.27.2. Definition — Reviewed candidate

Patient Transport coordinates the operational progression of movement requests, including the patient's applicable mobility constraints. Departure, transit and arrival checkpoints make the physical journey visible to participants. The responsibility supports internal and inter-facility movement without treating arrival as acceptance of clinical responsibility at the destination.

23.27.3. Boundary — retained v0.1 source explanation

Transport completion does not perform clinical handover or establish a care responsibility transition.

23.27.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Transport request and dispatch progression are substantive assigned operational coordination responsibilities.

23.27.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.27.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.27.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.27.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Patient Transport Progression (p. 637)	function	Canonical / Established
Service: Patient Transport Tracking Service (p. 638)	service	Canonical / Established
Process: Patient Transport Process (p. 638)	process	Canonical / Established
BEC CT3: Patient Transfer Coordination (p. 634)	capability CT3	Proposed / Proposed

23.27.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Support Logistics Management (p. 41)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed

23.27.10. BEC CT3: Patient Transfer Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-transfer-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Patient Transport	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.27.10.1. Capability Overview

Coordinate patient transport requests, dispatch and observed departure, transit and arrival milestones. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.27.10.2. Definition — Reviewed candidate

Coordinate patient transport requests, dispatch and observed departure, transit and arrival milestones.

23.27.10.3. Intent

Coordinate patient transport requests, dispatch and observed departure, transit and arrival milestones.

23.27.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.27.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.27.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-854FF619` (line 8298). Working locators are not identity aliases.

23.27.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Transport Request Ingestion (p. 635)	feature FT	Canonical / Proposed
Feature: Transport Dispatch & Progress Tracking (p. 636)	feature FT	Canonical / Proposed

23.27.10.8. Feature: Transport Request Ingestion

Property	Value	Meaning
Reference Id	`FEAT-HSO-24`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION.FEAT-HSO-24`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Patient Transport → BEC CT3: Patient Transfer Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.FEAT-HSO-24`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Patient Transport	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:545 (p. 1378)	Source standing does not approve the derived assertion.

23.27.10.8.1. Feature Overview

Transport ingestion establishes the requested movement and applicable clinical mobility constraints. It gives operational coordination the context needed to arrange transit without making the request itself a departure, arrival or accepted care transition.

23.27.10.8.2. Definition / governed semantic detail

Ingest internal (portering) and external (ambulance, inter-hospital) patient transfer requests with clinical mobility requirements.

23.27.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.27.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Patient Transport Progression (p. 637)	Established

23.27.10.9. Feature: Transport Dispatch & Progress Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-25`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION.FEAT-HSO-25`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Patient Transport → BEC CT3: Patient Transfer Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.FEAT-HSO-25`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Patient Transport	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:546 (p. 1378)	Source standing does not approve the derived assertion.

23.27.10.9.1. Feature Overview

Transport progression connects dispatch, departure, transit and arrival evidence for a physical patient journey. The checkpoints make movement intelligible while preserving that reaching a destination does not establish clinical handover or assumption of responsibility there.

23.27.10.9.2. Definition / governed semantic detail

Dispatch transport orders, tracking departure, transit, and destination arrival checkpoints.

23.27.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.27.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Patient Transport Progression (p. 637)	Established
From: associated feature	Service: Patient Transport Tracking Service (p. 638)	Established

23.27.11. Function: Manage Patient Transport Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-patient-transport-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.FN-MANAGE-PATIENT-TRANSPORT-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Patient Transport	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.FN-MANAGE-PATIENT-TRANSPORT-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:253 (p. 1462)	Source standing does not approve the derived assertion.

23.27.11.1. Definition / governed semantic detail

Coordinates intra-facility and inter-facility patient movements: *Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Handover Completed*.

Additional Feature responsibility: `FEAT-HSO-24 — Transport Request Ingestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.

23.27.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Transport Dispatch & Progress Tracking (p. 636)	Established
To: realises feature	Feature: Transport Request Ingestion (p. 635)	Established
To: governed process	Process: Patient Transport Process (p. 638)	Established

23.27.12. Service: Patient Transport Tracking Service

Property	Value	Meaning
Reference Id	`HCM-SV-patient-transport-tracking-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.SV-PATIENT-TRANSPORT-TRACKING-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Patient Transport	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.SV-PATIENT-TRANSPORT-TRACKING-SERVICE`	Direct to this identity; never reusable.
Evidence	hso:254 (p. 1462)	Source standing does not approve the derived assertion.

23.27.12.1. Definition / governed semantic detail

Provides real-time transit telemetry.

Additional Feature responsibility: `FEAT-HSO-24 — Transport Request Ingestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.

23.27.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Transport Dispatch & Progress Tracking (p. 636)	Established

23.27.13. Process: Patient Transport Process

Property	Value	Meaning
Reference Id	`HCM-PR-patient-transport-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.PR-PATIENT-TRANSPORT-PROCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Patient Transport	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.PR-PATIENT-TRANSPORT-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:233 (p. 1419); processes:234 (p. 1419)	Source standing does not approve the derived assertion.

23.27.13.1. Process Overview

Patient transport progression records the physical journey from a movement request through departure, transit and destination arrival. Its checkpoints allow operational participants to follow the journey with relevant mobility context intact. Arrival closes a movement concern only under its applicable scope; it does not establish clinical handover or destination acceptance of care responsibility.

23.27.13.2. Definition / governed semantic detail

- **Owning Capability**: `PatientTransport`
- **Process Purpose**: Governs the physical portering and transport of patients between hospital departments, wards, diagnostic suites, and external facilities.
- **State Progression Lifecycle**: Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Clinical Handover Complete
- **Key State Dispositions**:
  - `TransportRequested`: Clinical unit books wheelchair, bed, or ambulance transport.
  - `PorterDispatched`: Wardsperson assigned and en route to pickup location.
  - `PatientCollected`: Patient identity verified and transfer initiated.
  - `In-Transit`: Patient actively moving through facility corridors or ambulance transit.
  - `DeliveredatDestination`: Patient arrives at target diagnostic suite or ward.
  - `ClinicalHandoverComplete`: Safe custody and clinical briefing transferred to receiving staff.

23.27.13.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Patient Transport Progression (p. 637)	Established

23.28. BEC CT2: Clinical Logistics Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-logistics-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:466 (p. 1375); enabling:548 (p. 1378); hso:264 (p. 1463)	Source standing does not approve the derived assertion.

23.28.1. Capability Overview

Coordinate specimen metadata and specimen movement traceability across responsible collection, courier and diagnostic services.

23.28.2. Definition — Reviewed candidate

Coordinate specimen metadata and specimen movement traceability across responsible collection, courier and diagnostic services.

23.28.3. Boundary — retained v0.1 source explanation

Tracking material movement does not perform diagnostic interpretation or establish general enterprise supply-chain ownership.

23.28.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Custody and transport information materially support diagnostic continuity and appropriate interpretation of specimen handling.

23.28.5. Boundary

Includes specimen metadata and specimen movement traceability. Supply/materiel movement belongs to Health Materiel Traceability Administration.

23.28.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.28.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.28.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Service: Specimen Transit Tracking Service (p. 644)	service	Canonical / Established
BEC CT3: Specimen Metadata Management (p. 641)	capability CT3	Proposed / Proposed
BEC CT3: Specimen Movement Traceability (p. 642)	capability CT3	Proposed / Proposed

23.28.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Support Logistics Management (p. 41)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed

23.28.10. BEC CT3: Specimen Metadata Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-specimen-metadata-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.CT3-BEC-SPECIMEN-METADATA-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.28.10.1. Capability Overview

Manage the identifying, descriptive, custody and relevant handling information needed to interpret a transported specimen. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.28.10.2. Definition — Reviewed candidate

Manage the identifying, descriptive, custody and relevant handling information needed to interpret a transported specimen.

23.28.10.3. Intent

Manage the identifying, descriptive, custody and relevant handling information needed to interpret a transported specimen.

23.28.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.28.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.28.11. BEC CT3: Specimen Movement Traceability

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-specimen-movement-traceability`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.CT3-BEC-SPECIMEN-MOVEMENT-TRACEABILITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.28.11.1. Capability Overview

Maintain attributable pickup, custody, transport-condition and delivery traceability for clinical specimens. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.28.11.2. Definition — Reviewed candidate

Maintain attributable pickup, custody, transport-condition and delivery traceability for clinical specimens.

23.28.11.3. Intent

Maintain attributable pickup, custody, transport-condition and delivery traceability for clinical specimens.

23.28.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.28.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.28.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-999FB8AB` (line 8397). Working locators are not identity aliases.

23.28.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Pathology Specimen Transport Tracking (p. 643)	feature FT	Canonical / Proposed

23.28.11.8. Feature: Pathology Specimen Transport Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-26`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.CT3-BEC-SPECIMEN-MOVEMENT-TRACEABILITY.FEAT-HSO-26`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Logistics Coordination → BEC CT3: Specimen Movement Traceability	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.FEAT-HSO-26`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Logistics Coordination	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:550 (p. 1378)	Source standing does not approve the derived assertion.

23.28.11.8.1. Feature Overview

Specimen transport tracking retains pickup, custody and condition context through laboratory delivery. It supports interpretation of the material's journey without treating a delivered package as proof of specimen suitability, completed analysis or an authorised diagnostic report.

23.28.11.8.2. Definition / governed semantic detail

Track pickup, custody chain, transport conditions, and laboratory delivery of clinical specimens.

23.28.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.28.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: associated feature	Service: Specimen Transit Tracking Service (p. 644)	Established

23.28.12. Service: Specimen Transit Tracking Service

Property	Value	Meaning
Reference Id	`HCM-SV-specimen-transit-tracking-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.SV-SPECIMEN-TRANSIT-TRACKING-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.SV-SPECIMEN-TRANSIT-TRACKING-SERVICE`	Direct to this identity; never reusable.
Evidence	hso:268 (p. 1463)	Source standing does not approve the derived assertion.

23.28.12.1. Definition / governed semantic detail

Exposes chain-of-custody tracking.

23.28.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Pathology Specimen Transport Tracking (p. 643)	Established

23.29. BEC CT2: Discharge Management

Property	Value	Meaning
Reference Id	`HCM-BEC-discharge-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:467 (p. 1375); enabling:552 (p. 1378); hso:275 (p. 1463)	Source standing does not approve the derived assertion.

23.29.1. Capability Overview

Discharge Management coordinates the reported milestones needed around discharge, including medication supply, transport and home-care confirmations. It makes readiness and impediments visible across disciplines and communicates departure information when physical exit occurs. The responsibility preserves a distinction between gathering confirmations and making the clinical decision that discharge is appropriate.

23.29.2. Definition — Reviewed candidate

Discharge Management coordinates the reported milestones needed around discharge, including medication supply, transport and home-care confirmations. It makes readiness and impediments visible across disciplines and communicates departure information when physical exit occurs. The responsibility preserves a distinction between gathering confirmations and making the clinical decision that discharge is appropriate.

23.29.3. Boundary — retained v0.1 source explanation

Milestone completion does not transfer clinical responsibility or prove the receiving provider accepted it. Readiness, exit and clinical decision authority remain distinct.

23.29.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Cross-team milestone coordination and departure notification are substantive assigned platform responsibilities.

23.29.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

23.29.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

23.29.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.29.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Discharge Coordination Progression (p. 651)	function	Canonical / Established
Service: Discharge Readiness Telemetry (p. 652)	service	Canonical / Established
Process: Discharge Progression Process (p. 652)	process	Canonical / Established
BEC CT3: Discharge Progression & Coordination (p. 647)	capability CT3	Proposed / Proposed

23.29.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Coordination & Transition (p. 37)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Activity & Execution (p. 857)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT3: Entity-Relative Operational Progression (p. 861)	Proposed

23.29.10. BEC CT3: Discharge Progression & Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-discharge-progression-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

23.29.10.1. Capability Overview

Coordinate externally established discharge milestones, multidisciplinary readiness information and departure notifications. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

23.29.10.2. Definition — Reviewed candidate

Coordinate externally established discharge milestones, multidisciplinary readiness information and departure notifications.

23.29.10.3. Intent

Coordinate externally established discharge milestones, multidisciplinary readiness information and departure notifications.

23.29.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

23.29.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

23.29.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-3414F1EB` (line 8471). Working locators are not identity aliases.

23.29.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Discharge Milestone Tracking (p. 648)	feature FT	Canonical / Proposed
Feature: Discharge Readiness & Coordination Oversight (p. 649)	feature FT	Canonical / Proposed
Feature: Discharge Notification Dispatch (p. 650)	feature FT	Canonical / Proposed

23.29.10.8. Feature: Discharge Milestone Tracking

Property	Value	Meaning
Reference Id	`FEAT-HSO-27`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-27`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management → BEC CT3: Discharge Progression & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FEAT-HSO-27`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Discharge Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:554 (p. 1378)	Source standing does not approve the derived assertion.

23.29.10.8.1. Feature Overview

Discharge milestone tracking records confirmations contributed by the relevant clinical and operational participants. It makes outstanding requirements visible without allowing the tracking mechanism to declare clinical readiness or assume that every local milestone defines a universal discharge pathway.

23.29.10.8.2. Definition / governed semantic detail

Track clinical criteria-led discharge milestones (medications dispensed, transportation arranged, home care confirmed).

23.29.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.29.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Discharge Coordination Progression (p. 651)	Established

23.29.10.9. Feature: Discharge Readiness & Coordination Oversight

Property	Value	Meaning
Reference Id	`FEAT-HSO-28`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-28`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management → BEC CT3: Discharge Progression & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FEAT-HSO-28`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Discharge Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:555 (p. 1378)	Source standing does not approve the derived assertion.

23.29.10.9.1. Feature Overview

Readiness oversight brings multidisciplinary, home-care and transport confirmations into one coordination view. Aggregation supports the responsible participants' decisions but does not itself confer authority to discharge or establish acceptance of continuing care by another provider.

23.29.10.9.2. Definition / governed semantic detail

Track and aggregate multidisciplinary discharge milestones, home care confirmations, and transit readiness across care teams.

23.29.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.29.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Discharge Coordination Progression (p. 651)	Established
From: associated feature	Service: Discharge Readiness Telemetry (p. 652)	Established

23.29.10.10. Feature: Discharge Notification Dispatch

Property	Value	Meaning
Reference Id	`FEAT-HSO-29`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-29`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management → BEC CT3: Discharge Progression & Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FEAT-HSO-29`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Discharge Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:556 (p. 1378)	Source standing does not approve the derived assertion.

23.29.10.10.1. Feature Overview

Departure notification communicates the physical exit and associated record and place-state context to the applicable participants. It distinguishes departure information from earlier readiness and from the clinical decision, while retaining the separate question of responsibility actually assumed elsewhere.

23.29.10.10.2. Definition / governed semantic detail

Dispatch departure notifications, updated bed states, and finalized discharge summaries to external care providers upon physical patient exit.

23.29.10.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

23.29.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Discharge Coordination Progression (p. 651)	Established

23.29.11. Function: Manage Discharge Coordination Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-discharge-coordination-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.FN-MANAGE-DISCHARGE-COORDINATION-PROGRESSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FN-MANAGE-DISCHARGE-COORDINATION-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:279 (p. 1463); hso:282 (p. 1463)	Source standing does not approve the derived assertion.

23.29.11.1. Definition / governed semantic detail

Synchronises multi-agency discharge readiness (**illustrative preparation; publication, authorisation and confirmed exit are distinct**): *Discharge Planning Initiated → Medications Reconciled → Transport Booked → Discharge Summary Finalised → Physically Departed*.

23.29.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Discharge Readiness & Coordination Oversight (p. 649)	Established
To: realises feature	Feature: Discharge Milestone Tracking (p. 648)	Established
To: realises feature	Feature: Discharge Notification Dispatch (p. 650)	Established
To: governed process	Process: Discharge Progression Process (p. 652)	Established

23.29.12. Service: Discharge Readiness Telemetry

Property	Value	Meaning
Reference Id	`HCM-SV-discharge-readiness-telemetry`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.SV-DISCHARGE-READINESS-TELEMETRY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.SV-DISCHARGE-READINESS-TELEMETRY`	Direct to this identity; never reusable.
Evidence	hso:283 (p. 1463); hso:282 (p. 1463)	Source standing does not approve the derived assertion.

23.29.12.1. Definition / governed semantic detail

Discloses discharge barrier checklists and planned departure times.

23.29.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Discharge Readiness & Coordination Oversight (p. 649)	Established

23.29.13. Process: Discharge Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-discharge-progression-process`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.PR-DISCHARGE-PROGRESSION-PROCESS`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Service Operations Enablement → BEC CT2: Discharge Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.PR-DISCHARGE-PROGRESSION-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:263 (p. 1420); processes:264 (p. 1420)	Source standing does not approve the derived assertion.

23.29.13.1. Process Overview

Discharge progression coordinates the confirmations and practical requirements around leaving care. Clinical readiness, medications, destination arrangements and transport are related but distinct contributions, and the source progression retains its stated local applicability. Operational coordination does not originate the clinical discharge decision or prove that another provider assumed responsibility.

23.29.13.2. Definition / governed semantic detail

- **Owning Capability**: `DischargeManagement`
- **Process Purpose**: Governs the multidisciplinary coordination of patient discharge planning, pharmacy reconciliation, transport, and community handover.
- **State Progression Lifecycle**: Discharge Planning Initiated → Clinical Readiness Confirmed → Medications Reconciled → Transport & Services Booked → Discharge Summary Signed → Physically Departed (Illustrative preparation; publication, authorisation and exit distinct)
- **Key State Dispositions**:
  - `DischargePlanningInitiated`: Estimated Date of Discharge (EDD) set on admission.
  - `ClinicalReadinessConfirmed`: Attending medical team declares patient fit for discharge.
  - `MedicationsReconciled`: Hospital pharmacy reconciles and dispenses discharge medications.
  - `Transport&ServicesBooked`: Community nursing, home equipment, and patient transport booked.
  - `DischargeSummarySigned`: Applicable signed summary is available/published and may be communicated before departure where the Business behaviour requires it; signing, finalisation, publication and discharge authorisation remain distinct.
  - `PhysicallyDeparted`: Confirmed physical exit triggers departure notifications, updated bed-state communication and finalised-summary dispatch to external care providers under `FEAT-HSO-29`; care-place turnover follows its own responsibility.
- **Timing reconciliation**: Preparation, information availability/publication, applicable discharge authorisation and physical exit are distinct. Earlier preparation/communication is permitted; it does not realise a Feature triggered by confirmed exit. This illustrated local pathway establishes neither universal publication timing nor a universal discharge state machine, and adds no inferred publication event or authorisation state.
- **Preserved uncertainty**: Summary signing and finalisation are not universal equivalents. Detailed applicability, discharge-authority workflows and timing beyond the established preparation/confirmed-exit distinction remain unestablished.

23.29.13.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: governed process	Function: Manage Discharge Coordination Progression (p. 651)	Established

24. BEC CT1: Shared Information & Coordination Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-governed-health-information-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT`	Current proposed context where marked Proposed.
Name history	Governed Health Information Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668	Source standing does not approve the derived assertion.

24.1. Capability Overview

Enable governed shared health-information use, exchange, stewardship, knowledge, collaboration, external work coordination, calendars, presentation and information design.

24.2. Definition — Reviewed candidate

Enable governed shared health-information use, exchange, stewardship, knowledge, collaboration, external work coordination, calendars, presentation and information design.

24.3. Boundary

Preservation, custody, discoverability and presentation do not confer originating fact authority. Clinical documentation administration and clinical-context use retain their own responsibilities.

24.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668

24.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Preserve, find, exchange, qualify and protect health information and the governed definitions needed to interpret it.

Previous overview: This frame makes a managed record usable over time and across authorised purposes. It separates clinical synthesis, binary-content association, external exchange, semantic definitions, privacy and evidence so that none becomes an undifferentiated information utility.

Previous intent: Keep managed health information usable and attributable across time, boundaries and authorised purposes.

24.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Patient Clinical Record (p. 656)	capability CT2	Canonical / Proposed
BEC CT2: Health Information Exchange Enablement (p. 686)	capability CT2	Canonical / Proposed
BEC CT2: Health Information Access (p. 701)	capability CT2	Canonical / Proposed
BEC CT2: Health Information Control (p. 715)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Knowledge Services (p. 742)	capability CT2	Canonical / Proposed
BEC CT2: Clinical Collaboration (p. 756)	capability CT2	Canonical / Proposed
BEC CT2: General Collaboration (p. 768)	capability CT2	Canonical / Proposed
BEC CT2: Calendar Management (p. 779)	capability CT2	Canonical / Proposed
BEC CT2: Presentation Services (p. 786)	capability CT2	Canonical / Proposed
BEC CT2: Information Design Governance (p. 792)	capability CT2	Canonical / Proposed
BEC CT2: Health Information Communication (p. 710)	capability CT2	Proposed / Proposed
BEC CT2: Information Asset Stewardship Management (p. 728)	capability CT2	Proposed / Proposed
BEC CT2: Information Content Maintainer Management (p. 731)	capability CT2	Proposed / Proposed
BEC CT2: Workflow & Activity Coordination (p. 773)	capability CT2	Proposed / Proposed

24.9. BEC CT2: Patient Clinical Record

Property	Value	Meaning
Reference Id	`HCM-BEC-patient-clinical-record`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:565 (p. 1379); enabling:608 (p. 1380); ise:35 (p. 1466)	Source standing does not approve the derived assertion.

24.9.1. Capability Overview

Patient Clinical Record provides a governed longitudinal representation across encounters, providers and source systems. Assembly supplies the timeline, active access supports current care context, and durable preservation retains the history of what Harmonia received, knew or managed. These contributions are complementary: a responsive working view must not become the authority for durable history.

24.9.2. Definition — Reviewed candidate

Patient Clinical Record provides a governed longitudinal representation across encounters, providers and source systems. Assembly supplies the timeline, active access supports current care context, and durable preservation retains the history of what Harmonia received, knew or managed. These contributions are complementary: a responsive working view must not become the authority for durable history.

24.9.3. Intent — retained v0.1 source explanation

Make cross-institutional information usable over time without erasing source provenance or prior knowledge through correction.

24.9.4. Boundary — retained v0.1 source explanation

Preservation does not originate clinical facts. Active distributed access and authoritative durable preservation remain separate under AX-05.

24.9.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Longitudinal assembly, active access and governed durable history are direct information-management responsibilities within the Core boundary.

24.9.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.9.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.9.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.9.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Primary Care Coordination (p. 669)	capability CT3	Canonical / Proposed
BEC CT3: Remote Patient Monitoring (p. 675)	capability CT3	Canonical / Proposed
Function: Assemble Longitudinal Clinical Record (p. 681)	function	Canonical / Established
Service: Longitudinal Clinical Record Query (p. 682)	service	Canonical / Established
Function: Maintain Active Clinical Record (p. 683)	function	Canonical / Established
Service: Active Problem & Allergy Summary Query (p. 684)	service	Canonical / Established
Function: Preserve Durable Clinical Record (p. 684)	function	Canonical / Established
Service: Historical Clinical Record Access (p. 685)	service	Canonical / Established
BEC CT3: Linked Information Curation (p. 663)	capability CT3	Proposed / Proposed
BEC CT3: Longitudinal Clinical Record Administration (p. 658)	capability CT3	Proposed / Proposed

24.9.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT1: Individual Care Delivery (p. 5)	Proposed
From: enabled by	BC CT2: Health Information Management (p. 105)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed
To: enterprise enabled by	Enterprise CT3: Active Information Availability (p. 846)	Proposed
To: enterprise enabled by	Enterprise CT3: Authoritative Durable State Establishment (p. 847)	Proposed

24.9.11. BEC CT3: Longitudinal Clinical Record Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-longitudinal-clinical-record-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.9.11.1. Capability Overview

Administer longitudinal record assembly, active clinical summaries and durable received/managed history with source attribution and supersession. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.9.11.2. Definition — Reviewed candidate

Administer longitudinal record assembly, active clinical summaries and durable received/managed history with source attribution and supersession.

24.9.11.3. Intent

Administer longitudinal record assembly, active clinical summaries and durable received/managed history with source attribution and supersession.

24.9.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.9.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.9.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-15AB949F` (line 8615). Working locators are not identity aliases.

24.9.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Longitudinal Health Record (LHR) Assembly (p. 659)	feature FT	Canonical / Proposed
Feature: Active Clinical Record Access (p. 660)	feature FT	Canonical / Proposed
Feature: Durable Clinical Record Preservation (p. 661)	feature FT	Canonical / Proposed
Feature: Clinical Information Historical Reconstruction (p. 662)	feature FT	Proposed / Proposed

24.9.11.8. Feature: Longitudinal Health Record (LHR) Assembly

Property	Value	Meaning
Reference Id	`FEAT-ISE-01`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-01`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Longitudinal Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-ISE-01`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Patient Clinical Record	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:610 (p. 1380)	Source standing does not approve the derived assertion.

24.9.11.8.1. Feature Overview

Longitudinal assembly relates contributions from disparate systems into a usable chronology. The unified view preserves source and subject context rather than turning consolidation into originating clinical authority or erasing differences in the standing of individual assertions.

24.9.11.8.2. Definition / governed semantic detail

Assemble disparate encounters, diagnostic reports, medications, and clinical documents into a unified, vendor-neutral longitudinal timeline.

24.9.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.9.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Assemble Longitudinal Clinical Record (p. 681)	Established
From: associated feature	Service: Longitudinal Clinical Record Query (p. 682)	Established

24.9.11.9. Feature: Active Clinical Record Access

Property	Value	Meaning
Reference Id	`FEAT-ISE-02`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-02`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Longitudinal Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-ISE-02`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Patient Clinical Record	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:611 (p. 1380)	Source standing does not approve the derived assertion.

24.9.11.9.1. Feature Overview

Active access supports responsive use of consolidated clinical summaries in ongoing care. The working representation remains reconstructable active state; speed and availability do not promote it to authoritative durable truth when persistence or convergence is unavailable.

24.9.11.9.2. Definition / governed semantic detail

Provide low-latency, active distributed access to consolidated patient clinical summaries.

24.9.11.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.9.11.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Maintain Active Clinical Record (p. 683)	Established
From: associated feature	Service: Active Problem & Allergy Summary Query (p. 684)	Established

24.9.11.10. Feature: Durable Clinical Record Preservation

Property	Value	Meaning
Reference Id	`FEAT-ISE-03`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-03`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Longitudinal Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-ISE-03`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Patient Clinical Record	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:612 (p. 1380)	Source standing does not approve the derived assertion.

24.9.11.10.1. Feature Overview

Durable preservation retains the governed longitudinal representation and the history of what Harmonia received or managed. Corrections establish changed knowledge without retrospec-

tively rewriting that history, and preservation neither originates clinical facts nor automatically establishes legal qualification of the record.

24.9.11.10.2. Definition / governed semantic detail

Durably maintain governed, vendor-neutral longitudinal clinical representation and preservation with verifiable integrity controls.

24.9.11.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.9.11.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Preserve Durable Clinical Record (p. 684)	Established
From: associated feature	Service: Historical Clinical Record Access (p. 685)	Established

24.9.11.11. Feature: Clinical Information Historical Reconstruction

Property	Value	Meaning
Reference Id	`HCM-FT-clinical-information-historical-reconstruction`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-STEP2-CLINICAL-INFORMATION-HISTORICAL-RECONSTRUCTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Longitudinal Clinical Record Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-STEP2-CLINICAL-INFORMATION-HISTORICAL-RECONSTRUCTION`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

24.9.11.11.1. Feature Overview

Current state and historical operational truth answer different questions. Reconstruction retains superseded and corrected knowledge without rewriting the earlier record.

24.9.11.11.2. Definition — Proposed

Reconstruct what information Harmonia received, knew or managed at a relevant historical basis.

24.9.11.11.3. Boundary

Does not infer that historical information was clinically true or that a present source state applied in the past.

24.9.11.11.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.9.11.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

24.9.12. BEC CT3: Linked Information Curation

Property	Value	Meaning
Reference Id	`HCM-BEC-associated-clinical-binary-content-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT`	Direct to this identity; never reusable.
Name history	Associated Clinical Binary Content Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	sylloge-binary-content-management.md:14; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.9.12.1. Capability Overview

Curate associations to information and binary content needed to interpret and use a clinical record, retaining custody and source-authority distinctions.

24.9.12.2. Definition — Reviewed candidate

Curate associations to information and binary content needed to interpret and use a clinical record, retaining custody and source-authority distinctions.

24.9.12.3. Boundary

Association is not physical custody. Persistence authority, source clinical meaning and external repository authority remain separate.

24.9.12.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.9.12.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

24.9.12.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task sylloge-binary-content-management.md:14; information-responsibility.md:10

24.9.12.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.9.12.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Associate, resolve, retrieve and assemble binary content needed to use managed clinical or operational information.

Previous overview: Logical information may refer to one or several payloads held internally or by an external custodian. This capability keeps association and custody knowledge explicit so that content can be obtained under the governing information contract.

Previous intent: Obtain the content needed to use a managed information item without assuming where every payload is physically held.

24.9.12.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Binary Content Resolution & Assembly (p. 665)	feature FT	Proposed / Proposed
Function: Resolve Associated Binary Content (p. 666)	function	Proposed / Proposed

Element / page	Type / tier	Element / placement status
Function: Assemble Associated Binary Content (p. 667)	function	Proposed / Proposed
Service: Associated Binary Content Retrieval (p. 668)	service	Proposed / Proposed

24.9.12.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Information Management (p. 105)	Proposed
To: enterprise enabled by	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed

24.9.12.11. Feature: Binary Content Resolution & Assembly

Property	Value	Meaning
Reference Id	`HCM-FT-binary-content-resolution-assembly`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FEAT-STEP2-BINARY-CONTENT-RESOLUTION-ASSEMBLY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Linked Information Curation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FEAT-STEP2-BINARY-CONTENT-RESOLUTION-ASSEMBLY`	Direct to this identity; never reusable.
Evidence	sylloge-binary-content-management.md:14	Source standing does not approve the derived assertion.

24.9.12.11.1. Feature Overview

A logical clinical item may reference externally held or multipart payloads. The Feature supplies a usable content result while retaining custody and missing-content standing.

24.9.12.11.2. Definition — Proposed

Resolve governed payload associations and retrieve or assemble the content required by a requesting information context.

24.9.12.11.3. Boundary

No assumption of local copying, shared storage, universal content retention or originating clinical authority.

24.9.12.11.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Associated Clinical Binary Content Enablement, rather than inheriting an adjacent responsibility.

24.9.12.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task sylloge-binary-content-management.md:14

24.9.12.11.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Resolve Associated Binary Content (p. 666)	Proposed
From: realises feature	Function: Assemble Associated Binary Content (p. 667)	Proposed

24.9.12.12. Function: Resolve Associated Binary Content

Property	Value	Meaning
Reference Id	`HCM-FN-resolve-associated-binary-content`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-RESOLVE-ASSOCIATED-BINARY-CONTENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Linked Information Curation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-RESOLVE-ASSOCIATED-BINARY-CONTENT`	Direct to this identity; never reusable.
Evidence	sylloge-binary-content-management.md:14	Source standing does not approve the derived assertion.

24.9.12.12.1. Function Overview

Changes an unresolved content request into an identified retrieval basis or an explicit inability to resolve it.

24.9.12.12.2. Definition — Proposed

Resolve logical content associations to an authorised, custody-qualified retrieval basis.

24.9.12.12.3. Boundary

Does not assert payload existence, copy all content or establish clinical meaning.

24.9.12.12.4. Proposed Placement Rationale

Behaviour remains within Associated Clinical Binary Content Enablement responsibility.

24.9.12.12.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task sylloge-binary-content-management.md:14

24.9.12.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Binary Content Resolution & Assembly (p. 665)	Proposed
From: uses function	Service: Associated Binary Content Retrieval (p. 668)	Proposed

24.9.12.13. Function: Assemble Associated Binary Content

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-associated-binary-content`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-ASSEMBLE-ASSOCIATED-BINARY-CONTENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Linked Information Curation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-ASSEMBLE-ASSOCIATED-BINARY-CONTENT`	Direct to this identity; never reusable.
Evidence	sylloge-binary-content-management.md:14	Source standing does not approve the derived assertion.

24.9.12.13.1. Function Overview

Moves a qualified retrieval request to an assembled result or explicit unavailable/partial standing.

24.9.12.13.2. Definition — Proposed

Retrieve and assemble the requested logical content from its permitted constituent payloads.

24.9.12.13.3. Boundary

No storage topology or universal local custody is selected.

24.9.12.13.4. Proposed Placement Rationale

Behaviour remains within Associated Clinical Binary Content Enablement responsibility.

24.9.12.13.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task sylloge-binary-content-management.md:14

24.9.12.13.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Binary Content Resolution & Assembly (p. 665)	Proposed
From: uses function	Service: Associated Binary Content Retrieval (p. 668)	Proposed

24.9.12.14. Service: Associated Binary Content Retrieval

Property	Value	Meaning
Reference Id	`HCM-SV-associated-binary-content-retrieval`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.SV-ASSOCIATED-BINARY-CONTENT-RETRIEVAL`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Linked Information Curation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.SV-ASSOCIATED-BINARY-CONTENT-RETRIEVAL`	Direct to this identity; never reusable.
Evidence	sylloge-binary-content-management.md:14	Source standing does not approve the derived assertion.

24.9.12.14.1. Service Overview

The client contract returns authorised content or explicit incomplete/unavailable standing while retaining custody attribution.

24.9.12.14.2. Definition — Proposed

Allow information-consuming capabilities to request resolution and retrieval of associated content.

24.9.12.14.3. Boundary

The client acquires neither physical repository access nor originating clinical authority.

24.9.12.14.4. Proposed Placement Rationale

Behaviour remains within Associated Clinical Binary Content Enablement responsibility.

24.9.12.14.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task sylloge-binary-content-management.md:14

24.9.12.14.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: uses function	Function: Resolve Associated Binary Content (p. 666)	Proposed
To: uses function	Function: Assemble Associated Binary Content (p. 667)	Proposed

24.9.13. BEC CT3: Primary Care Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-primary-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE`	Direct to this identity; never reusable.
Established name	Primary Care	Current display rename is Proposed; meaning unchanged.
Name history	Primary Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:362 (p. 1372); enabling:389 (p. 1373); sd:51 (p. 1449)	Source standing does not approve the derived assertion.

24.9.13.1. Capability Overview

Primary Care enables information continuity between general practice and other care settings through shared-record ingestion and notifications of acute events. A GP's notes, summaries

and immunisation records can contribute to the longitudinal view, while admission and discharge information can return to the registered practitioner. The enabling responsibility is the connection between contexts, rather than practice management or primary clinical care.

24.9.13.2. Boundary

Receiving a summary or dispatching a notification does not perform consultation, clinical review or follow-up.

24.9.13.3. Harmonia Architectural Relevance

These flows materially support continuity across primary and acute care without assigning general-practice authority to Harmonia.

24.9.13.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.9.13.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

24.9.13.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.9.13.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: General Practice Shared Record Ingestion (p. 671)	feature FT	Canonical / Proposed
Feature: Primary Care Notification Dispatch (p. 671)	feature FT	Canonical / Proposed
Function: Correlate Primary Care Consultation Context (p. 672)	function	Canonical / Established
Service: Primary Care Summary Ingress (p. 673)	service	Canonical / Established
Function: Coordinate Multi-Agency Chronic Care Plan (p. 674)	function	Canonical / Established
Service: Chronic Care Timeline Query (p. 674)	service	Canonical / Established

24.9.13.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

24.9.13.9. Feature: General Practice Shared Record Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SD-01`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FEAT-SD-01`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Primary Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FEAT-SD-01`	Direct to this identity; never reusable.
Evidence	enabling:391 (p. 1373)	Source standing does not approve the derived assertion.

24.9.13.9.1. Feature Overview

Shared-record ingestion brings general-practice contributions into cross-provider continuity. The summary or encounter note retains its clinical source while becoming available alongside regional information; consolidation does not transfer the GP's professional authority to the platform.

24.9.13.9.2. Definition / governed semantic detail

Ingest shared health summaries, encounter notes, and immunization records from primary care practice management systems.

24.9.13.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Correlate Primary Care Consultation Context (p. 672)	Established
From: associated feature	Service: Primary Care Summary Ingress (p. 673)	Established

24.9.13.10. Feature: Primary Care Notification Dispatch

Property	Value	Meaning
Reference Id	`FEAT-SD-02`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FEAT-SD-02`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Primary Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FEAT-SD-02`	Direct to this identity; never reusable.
Evidence	enabling:392 (p. 1373)	Source standing does not approve the derived assertion.

24.9.13.10.1. Feature Overview

Primary-care notifications inform the registered GP of significant acute-care events. This outward notification responsibility is distinct from querying a chronic-care timeline: the latter shares context but has no established equivalence to dispatching the required arrival, admission or discharge notice.

24.9.13.10.2. Definition / governed semantic detail

Dispatch emergency department arrival, acute admission, and discharge notifications to the patient's registered general practitioner.

24.9.13.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Coordinate Multi-Agency Chronic Care Plan (p. 674)	Unresolved association
From: associated feature	Service: Chronic Care Timeline Query (p. 674)	Unresolved association

24.9.13.11. Function: Correlate Primary Care Consultation Context

Property	Value	Meaning
Reference Id	`HCM-FN-correlate-primary-care-consultation-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FN-CORRELATE-PRIMARY-CARE-CONSULTATION-CONTEXT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Primary Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FN-CORRELATE-PRIMARY-CARE-CONSULTATION-CONTEXT`	Direct to this identity; never reusable.
Evidence	sd:55 (p. 1449)	Source standing does not approve the derived assertion.

24.9.13.11.1. Definition / governed semantic detail

Binds GP consultation notes, vital signs, and encounter episodes to the regional Longitudinal Health Record (LHR).

24.9.13.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: General Practice Shared Record Ingestion (p. 671)	Established

24.9.13.12. Service: Primary Care Summary Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-primary-care-summary-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.SV-PRIMARY-CARE-SUMMARY-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Primary Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.SV-PRIMARY-CARE-SUMMARY-INGRESS`	Direct to this identity; never reusable.
Evidence	sd:56 (p. 1449)	Source standing does not approve the derived assertion.

24.9.13.12.1. Definition / governed semantic detail

Ingests GP event summaries into the HIE.

24.9.13.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: General Practice Shared Record Ingestion (p. 671)	Established

24.9.13.13. Function: Coordinate Multi-Agency Chronic Care Plan

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-multi-agency-chronic-care-plan`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FN-COORDINATE-MULTI-AGENCY-CHRONIC-CARE-PLAN`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Primary Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FN-COORDINATE-MULTI-AGENCY-CHRONIC-CARE-PLAN`	Direct to this identity; never reusable.
Evidence	sd:59 (p. 1449); sd:58 (p. 1449)	Source standing does not approve the derived assertion.

24.9.13.13.1. Definition / governed semantic detail

Enables GPs to track multi-provider disease management milestones (e.g., Diabetes or Cardiovascular Care Plans).

Feature association not established: Strategy `FEAT-SD-02 — Primary Care Notification Dispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

24.9.13.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Primary Care Notification Dispatch (p. 671)	Unresolved association

24.9.13.14. Service: Chronic Care Timeline Query

Property	Value	Meaning
Reference Id	`HCM-SV-chronic-care-timeline-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.SV-CHRONIC-CARE-TIMELINE-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Primary Care Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.SV-CHRONIC-CARE-TIMELINE-QUERY`	Direct to this identity; never reusable.
Evidence	sd:60 (p. 1449); sd:58 (p. 1449)	Source standing does not approve the derived assertion.

24.9.13.14.1. Definition / governed semantic detail

Exposes aggregated multidisciplinary care timelines.

Feature association not established: Strategy `FEAT-SD-02 — Primary Care Notification Dispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

24.9.13.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Primary Care Notification Dispatch (p. 671)	Unresolved association

24.9.14. BEC CT3: Remote Patient Monitoring

Property	Value	Meaning
Reference Id	`HCM-BEC-remote-virtual-care`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE`	Direct to this identity; never reusable.

Property	Value	Meaning
Established name	Remote & Virtual Care	Current display rename is Proposed; meaning unchanged.
Name history	Remote & Virtual Care	Names do not establish another identity.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:376 (p. 1372); enabling:434 (p. 1375); sd:221 (p. 1454)	Source standing does not approve the derived assertion.

24.9.14.1. Capability Overview

Remote & Virtual Care integrates remotely obtained observations and associates virtual consultations with their patient and episode context. It distinguishes a telemetry contribution from the encounter in which a practitioner may interpret it. The enabling responsibility concerns information continuity across delivery modes, preserving the setting-independent meaning of the Business care capabilities.

24.9.14.2. Boundary — v0.1 explanation

Telemetry acquisition is not clinical monitoring judgement or treatment; session association does not establish clinical adequacy.

24.9.14.3. Harmonia Architectural Relevance

Remote data and virtual-session context materially extend information continuity across care locations and modes.

24.9.14.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.9.14.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

24.9.14.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.9.14.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Remote Telemetry Ingestion (p. 677)	feature FT	Canonical / Proposed
Feature: Virtual Care Session Context Binding (p. 678)	feature FT	Canonical / Proposed

Element / page	Type / tier	Element / placement status
Function: Correlate Virtual Care Consultation Context (p. 679)	function	Canonical / Established
Service: Virtual Encounter Context Resolution (p. 679)	service	Canonical / Established
Function: Ingest Remote Patient Telemetry (p. 680)	function	Canonical / Established
Service: Remote Telemetry Ingress (p. 681)	service	Canonical / Established

24.9.14.8. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Management (p. 844)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed

24.9.14.9. Feature: Remote Telemetry Ingestion

Property	Value	Meaning
Reference Id	`FEAT-SD-19`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-19`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Remote Patient Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-19`	Direct to this identity; never reusable.
Evidence	enabling:436 (p. 1375)	Source standing does not approve the derived assertion.

24.9.14.9.1. Feature Overview

Remote telemetry ingestion accepts measurements and patient-reported contributions from outside the usual care site. The information can inform subsequent monitoring and interpretation, but acquisition alone does not establish clinical significance or authorise an intervention.

24.9.14.9.2. Definition / governed semantic detail

Ingest home patient monitoring data, biometric telemetry, and patient-reported outcome measures (PROMs).

24.9.14.9.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Ingest Remote Patient Telemetry (p. 680)	Established
From: associated feature	Service: Remote Telemetry Ingress (p. 681)	Established

24.9.14.10. Feature: Virtual Care Session Context Binding

Property	Value	Meaning
Reference Id	`FEAT-SD-20`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-20`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Remote Patient Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-20`	Direct to this identity; never reusable.
Evidence	enabling:437 (p. 1375)	Source standing does not approve the derived assertion.

24.9.14.10.1. Feature Overview

Virtual-session binding associates a consultation with the enduring record and the relevant episode. It preserves the clinical context across delivery modes without turning the communication session itself into proof of clinical conclusions or adequacy of care.

24.9.14.10.2. Definition / governed semantic detail

Associate synchronous virtual/telehealth consultations with the longitudinal health record and active care episode.

24.9.14.10.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Correlate Virtual Care Consultation Context (p. 679)	Established
From: associated feature	Service: Virtual Encounter Context Resolution (p. 679)	Established

24.9.14.11. Function: Correlate Virtual Care Consultation Context

Property	Value	Meaning
Reference Id	`HCM-FN-correlate-virtual-care-consultation-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-CORRELATE-VIRTUAL-CARE-CONSULTATION-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Remote Patient Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-CORRELATE-VIRTUAL-CARE-CONSULTATION-CONTEXT`	Direct to this identity; never reusable.
Evidence	sd:225 (p. 1454)	Source standing does not approve the derived assertion.

24.9.14.11.1. Definition / governed semantic detail

Binds video consultation metadata, remote practitioner identity, and shared digital whiteboard artifacts to the encounter.

24.9.14.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Virtual Care Session Context Binding (p. 678)	Established

24.9.14.12. Service: Virtual Encounter Context Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-virtual-encounter-context-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-VIRTUAL-ENCOUNTER-CONTEXT-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Remote Patient Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-VIRTUAL-ENCOUNTER-CONTEXT-RESOLUTION`	Direct to this identity; never reusable.
Evidence	sd:226 (p. 1454)	Source standing does not approve the derived assertion.

24.9.14.12.1. Definition / governed semantic detail

Binds virtual session identifiers to patient encounters.

24.9.14.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Virtual Care Session Context Binding (p. 678)	Established

24.9.14.13. Function: Ingest Remote Patient Telemetry

Property	Value	Meaning
Reference Id	`HCM-FN-ingest-remote-patient-telemetry`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-INGEST-REMOTE-PATIENT-TELEMETRY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Remote Patient Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-INGEST-REMOTE-PATIENT-TELEMETRY`	Direct to this identity; never reusable.
Evidence	sd:228 (p. 1454)	Source standing does not approve the derived assertion.

24.9.14.13.1. Definition / governed semantic detail

Ingests continuous or periodic physiological measurements (e.g., blood pressure, pulse oximetry, continuous glucose) from home monitoring devices.

24.9.14.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Remote Telemetry Ingestion (p. 677)	Established

24.9.14.14. Service: Remote Telemetry Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-remote-telemetry-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-REMOTE-TELEMETRY-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record → BEC CT3: Remote Patient Monitoring	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-REMOTE-TELEMETRY-INGRESS`	Direct to this identity; never reusable.
Evidence	sd:229 (p. 1454)	Source standing does not approve the derived assertion.

24.9.14.14.1. Definition / governed semantic detail

Ingests remote patient data streams into the LHR.

24.9.14.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Remote Telemetry Ingestion (p. 677)	Established

24.9.15. Function: Assemble Longitudinal Clinical Record

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-longitudinal-clinical-record`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-ASSEMBLE-LONGITUDINAL-CLINICAL-RECORD`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FN-ASSEMBLE-LONGITUDINAL-CLINICAL-RECORD`	Direct to this identity; never reusable.
Evidence	ise:39 (p. 1466)	Source standing does not approve the derived assertion.

24.9.15.1. Definition / governed semantic detail

Synthesises clinical documents, encounters, diagnostic reports, and medication histories across disparate source systems into a unified chronological clinical view.

24.9.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Longitudinal Health Record (LHR) Assembly (p. 659)	Established

24.9.16. Service: Longitudinal Clinical Record Query

Property	Value	Meaning
Reference Id	`HCM-SV-longitudinal-clinical-record-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-LONGITUDINAL-CLINICAL-RECORD-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.SV-LONGITUDINAL-CLINICAL-RECORD-QUERY`	Direct to this identity; never reusable.
Evidence	ise:40 (p. 1466)	Source standing does not approve the derived assertion.

24.9.16.1. Definition / governed semantic detail

Discloses the comprehensive clinical timeline to authorised clinicians.

24.9.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Longitudinal Health Record (LHR) Assembly (p. 659)	Established

24.9.17. Function: Maintain Active Clinical Record

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-active-clinical-record`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-MAINTAIN-ACTIVE-CLINICAL-RECORD`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FN-MAINTAIN-ACTIVE-CLINICAL-RECORD`	Direct to this identity; never reusable.
Evidence	ise:42 (p. 1466)	Source standing does not approve the derived assertion.

24.9.17.1. Definition / governed semantic detail

Maintains active problem lists, current medication regimens, allergies, and open clinical alerts for active care coordination.

24.9.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Active Clinical Record Access (p. 660)	Established

24.9.18. Service: Active Problem & Allergy Summary Query

Property	Value	Meaning
Reference Id	`HCM-SV-active-problem-allergy-summary-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-ACTIVE-PROBLEM-ALLERGY-SUMMARY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.SV-ACTIVE-PROBLEM-ALLERGY-SUMMARY-QUERY`	Direct to this identity; never reusable.
Evidence	ise:43 (p. 1466)	Source standing does not approve the derived assertion.

24.9.18.1. Definition / governed semantic detail

Exposes immediate active clinical summaries.

24.9.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Active Clinical Record Access (p. 660)	Established

24.9.19. Function: Preserve Durable Clinical Record

Property	Value	Meaning
Reference Id	`HCM-FN-preserve-durable-clinical-record`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-PRESERVE-DURABLE-CLINICAL-RECORD`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FN-PRESERVE-DURABLE-CLINICAL-RECORD`	Direct to this identity; never reusable.
Evidence	ise:45 (p. 1466)	Source standing does not approve the derived assertion.

24.9.19.1. Definition / governed semantic detail

Preserves the governed longitudinal representation and durable, immutable, append-only history of what Harmonia received, knew, asserted or managed, with provenance and versioning. Corrections/supersession establish changed knowledge without retrospectively altering that history or acquiring originating clinical authority.

24.9.19.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Durable Clinical Record Preservation (p. 661)	Established

24.9.20. Service: Historical Clinical Record Access

Property	Value	Meaning
Reference Id	`HCM-SV-historical-clinical-record-access`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-HISTORICAL-CLINICAL-RECORD-ACCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Patient Clinical Record	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.SV-HISTORICAL-CLINICAL-RECORD-ACCESS`	Direct to this identity; never reusable.
Evidence	ise:46 (p. 1466)	Source standing does not approve the derived assertion.

24.9.20.1. Definition / governed semantic detail

Provides governed historical record retrieval for authorised clinical and legal review; preservation does not itself establish legal qualification or originating authority.

24.9.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Durable Clinical Record Preservation (p. 661)	Established

24.10. BEC CT2: Health Information Exchange Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-health-information-exchange`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT`	Current proposed context where marked Proposed.
Reviewed hierarchy wording	Health Information Exchange	Display qualification retained pending CM-R05 name adjudication.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT`	Direct to this identity; never reusable.
Established name	Health Information Exchange	Current display rename is Proposed; meaning unchanged.
Name history	Health Information Exchange; Health Information Exchange Enablement	Names do not establish another identity.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:566 (p. 1379); enabling:614 (p. 1380); ise:52 (p. 1466); business:1225 (p. 1358)	Source standing does not approve the derived assertion.

24.10.1. Capability Overview

As a BEC, Health Information Exchange provides reusable information-movement enablement through submission, retrieval mediation, addressed distribution and syndication. The models distinguish sending to a participant, requesting information, deliberately distributing to named recipients and publishing changes to matching subscribers. Their common responsibility is the exchange context, including routing and delivery state, rather than the business meaning of every payload.

24.10.2. Definition — Reviewed candidate

As a BEC, Health Information Exchange provides reusable information-movement enablement through submission, retrieval mediation, addressed distribution and syndication. The models distinguish sending to a participant, requesting information, deliberately distributing to named recipients and publishing changes to matching subscribers. Their common responsibility is the exchange context, including routing and delivery state, rather than the business meaning of every payload.

24.10.3. Boundary — retained v0.1 source explanation

The capability owns exchange transaction state without acquiring clinical-document ownership. Its name collision with the BC remains unresolved; exchange egress ends management of the emitted representation.

24.10.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Multi-party exchange and per-destination delivery tracking are substantive Core responsibilities, bounded by the external contract and originating information authority.

24.10.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.10.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.10.7. Proposed Naming Decision

HUMAN ADJUDICATION REQUIRED: reviewed name Health Information Exchange duplicates the Canonical BC name under CM-R05. The existing BEC display qualification is retained; reviewed_name records the exact task wording. Identities and BC/BEC semantics remain distinct.

24.10.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.10.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Receive Information Submission (p. 695)	function	Canonical / Established
Service: HIE Submission Gateway Service (p. 696)	service	Canonical / Established
Function: Mediate Information Retrieval (p. 697)	function	Canonical / Established
Service: Federated Health Information Query (p. 698)	service	Canonical / Established
Function: Distribute Addressed Information (p. 698)	function	Canonical / Established

Element / page	Type / tier	Element / placement status
Service: Addressed Document Delivery Service (p. 699)	service	Canonical / Established
Function: Syndicate Information Change (p. 700)	function	Canonical / Established
Service: Clinical Event Syndication Stream (p. 700)	service	Canonical / Established
BEC CT3: HIE Requirement Management (p. 688)	capability CT3	Proposed / Proposed
BEC CT3: HIE Implementation Management (p. 689)	capability CT3	Proposed / Proposed
BEC CT3: HIE Endpoint State Management (p. 694)	capability CT3	Proposed / Proposed

24.10.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Information Exchange (p. 136)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed

24.10.11. BEC CT3: HIE Requirement Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-hie-requirement-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-REQUIREMENT-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.10.11.1. Capability Overview

Manage the identified information-exchange requirements, interaction contracts and participant obligations applicable to a health-information exchange. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.10.11.2. Definition — Reviewed candidate

Manage the identified information-exchange requirements, interaction contracts and participant obligations applicable to a health-information exchange.

24.10.11.3. Intent

Manage the identified information-exchange requirements, interaction contracts and participant obligations applicable to a health-information exchange.

24.10.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.10.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.10.12. BEC CT3: HIE Implementation Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-hie-implementation-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.10.12.1. Capability Overview

Manage implementation information and progression for established exchange requirements, including submission, retrieval, distribution and syndication interactions. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.10.12.2. Definition — Reviewed candidate

Manage implementation information and progression for established exchange requirements, including submission, retrieval, distribution and syndication interactions.

24.10.12.3. Intent

Manage implementation information and progression for established exchange requirements, including submission, retrieval, distribution and syndication interactions.

24.10.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.10.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.10.12.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-0FADE98F` (line 8725). Working locators are not identity aliases.

24.10.12.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Direct Submission Ingestion & Delivery (p. 690)	feature FT	Canonical / Proposed
Feature: Clinical Record Retrieval Mediation (p. 691)	feature FT	Canonical / Proposed
Feature: Addressed Clinical Distribution (p. 692)	feature FT	Canonical / Proposed
Feature: Event-Driven Syndication (p. 693)	feature FT	Canonical / Proposed

24.10.12.8. Feature: Direct Submission Ingestion & Delivery

Property	Value	Meaning
Reference Id	`FEAT-ISE-04`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-04`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement → BEC CT3: HIE Implementation Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-04`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Exchange Enablement	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:616 (p. 1380)	Source standing does not approve the derived assertion.

24.10.12.8.1. Feature Overview

Direct submission supports a sender's deliberate push of information across an exchange boundary. It preserves the distinction between ingress or delivery acceptance and the recipient's business handling, under the actual contract rather than an assumed meaning of transport success.

24.10.12.8.2. Definition / governed semantic detail

Support push submissions of clinical documents and messages across boundaries.

24.10.12.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.10.12.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Receive Information Submission (p. 695)	Established
From: associated feature	Service: HIE Submission Gateway Service (p. 696)	Established

24.10.12.9. Feature: Clinical Record Retrieval Mediation

Property	Value	Meaning
Reference Id	`FEAT-ISE-05`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-05`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement → BEC CT3: HIE Implementation Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-05`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Exchange Enablement	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:617 (p. 1380)	Source standing does not approve the derived assertion.

24.10.12.9.1. Feature Overview

Retrieval mediation connects a request with information held in participating repositories. It makes distributed retrieval possible without transferring repository authority, acquiring the clinical payload or treating a resolvable request as automatic permission to disclose.

24.10.12.9.2. Definition / governed semantic detail

Mediate query-based retrieval requests across external and internal repositories.

24.10.12.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.10.12.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Mediate Information Retrieval (p. 697)	Established
From: associated feature	Service: Federated Health Information Query (p. 698)	Established

24.10.12.10. Feature: Addressed Clinical Distribution

Property	Value	Meaning
Reference Id	`FEAT-ISE-06`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-06`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement → BEC CT3: HIE Implementation Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-06`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Exchange Enablement	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:618 (p. 1380)	Source standing does not approve the derived assertion.

24.10.12.10.1. Feature Overview

Addressed distribution sends information to explicitly designated recipients and tracks each destination's delivery separately. This preserves partial outcomes in a fan-out rather than collapsing several deliveries into a single success that hides a missing recipient.

24.10.12.10.2. Definition / governed semantic detail

Fan out clinical events and diagnostic reports to designated external and internal recipients with per-destination delivery tracking.

24.10.12.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.10.12.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Distribute Addressed Information (p. 698)	Established
From: associated feature	Service: Addressed Document Delivery Service (p. 699)	Established

24.10.12.11. Feature: Event-Driven Syndication

Property	Value	Meaning
Reference Id	`FEAT-ISE-07`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-07`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement → BEC CT3: HIE Implementation Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-07`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Exchange Enablement	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:619 (p. 1380)	Source standing does not approve the derived assertion.

24.10.12.11.1. Feature Overview

Syndication connects published changes with subscribers whose registered interests match them. It differs from named-recipient distribution in how consumers are selected, while retaining governed delivery and the distinction between notification and the subscriber's eventual business action.

24.10.12.11.2. Definition / governed semantic detail

Publish clinical and directory change events to subscribed systems matching interest filters.

24.10.12.11.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.10.12.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Syndicate Information Change (p. 700)	Established
From: associated feature	Service: Clinical Event Syndication Stream (p. 700)	Established

24.10.13. BEC CT3: HIE Endpoint State Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-hie-endpoint-state-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-ENDPOINT-STATE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.10.13.1. Capability Overview

Manage recognised exchange endpoint registrations, operational standing and contract-qualified availability information. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.10.13.2. Definition — Reviewed candidate

Manage recognised exchange endpoint registrations, operational standing and contract-qualified availability information.

24.10.13.3. Intent

Manage recognised exchange endpoint registrations, operational standing and contract-qualified availability information.

24.10.13.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.10.13.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.10.14. Function: Receive Information Submission

Property	Value	Meaning
Reference Id	`HCM-FN-receive-information-submission`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-RECEIVE-INFORMATION-SUBMISSION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-RECEIVE-INFORMATION-SUBMISSION`	Direct to this identity; never reusable.
Evidence	ise:57 (p. 1466)	Source standing does not approve the derived assertion.

24.10.14.1. Definition / governed semantic detail

Validates and ingests standards-compliant clinical messages from external health providers.

24.10.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Direct Submission Ingestion & Delivery (p. 690)	Established

24.10.15. Service: HIE Submission Gateway Service

Property	Value	Meaning
Reference Id	`HCM-SV-hie-submission-gateway-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-HIE-SUBMISSION-GATEWAY-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-HIE-SUBMISSION-GATEWAY-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:58 (p. 1466)	Source standing does not approve the derived assertion.

24.10.15.1. Definition / governed semantic detail

Ingress endpoint for regional data contributions.

24.10.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Direct Submission Ingestion & Delivery (p. 690)	Established

24.10.16. Function: Mediate Information Retrieval

Property	Value	Meaning
Reference Id	`HCM-FN-mediate-information-retrieval`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-MEDIATE-INFORMATION-RETRIEVAL`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-MEDIATE-INFORMATION-RETRIEVAL`	Direct to this identity; never reusable.
Evidence	ise:60 (p. 1466)	Source standing does not approve the derived assertion.

24.10.16.1. Definition / governed semantic detail

Federates queries across participating hospital and diagnostic node repositories.

24.10.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinical Record Retrieval Mediation (p. 691)	Established

24.10.17. Service: Federated Health Information Query

Property	Value	Meaning
Reference Id	`HCM-SV-federated-health-information-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-FEDERATED-HEALTH-INFORMATION-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-FEDERATED-HEALTH-INFORMATION-QUERY`	Direct to this identity; never reusable.
Evidence	ise:61 (p. 1466)	Source standing does not approve the derived assertion.

24.10.17.1. Definition / governed semantic detail

Resolves distributed record queries.

24.10.17.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Clinical Record Retrieval Mediation (p. 691)	Established

24.10.18. Function: Distribute Addressed Information

Property	Value	Meaning
Reference Id	`HCM-FN-distribute-addressed-information`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-DISTRIBUTE-ADDRESSED-INFORMATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-DISTRIBUTE-ADDRESSED-INFORMATION`	Direct to this identity; never reusable.
Evidence	ise:63 (p. 1466)	Source standing does not approve the derived assertion.

24.10.18.1. Definition / governed semantic detail

Directs point-to-point clinical documents to specific recipient practitioner endpoints.

24.10.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Addressed Clinical Distribution (p. 692)	Established

24.10.19. Service: Addressed Document Delivery Service

Property	Value	Meaning
Reference Id	`HCM-SV-addressed-document-delivery-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-ADDRESSED-DOCUMENT-DELIVERY-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-ADDRESSED-DOCUMENT-DELIVERY-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:64 (p. 1466)	Source standing does not approve the derived assertion.

24.10.19.1. Definition / governed semantic detail

Securely delivers documents to designated recipient mailboxes.

24.10.19.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Addressed Clinical Distribution (p. 692)	Established

24.10.20. Function: Syndicate Information Change

Property	Value	Meaning
Reference Id	`HCM-FN-syndicate-information-change`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-SYNDICATE-INFORMATION-CHANGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-SYNDICATE-INFORMATION-CHANGE`	Direct to this identity; never reusable.
Evidence	ise:66 (p. 1466)	Source standing does not approve the derived assertion.

24.10.20.1. Definition / governed semantic detail

Broadcasts event notifications and record updates to subscribed regional care network systems.

24.10.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Event-Driven Syndication (p. 693)	Established

24.10.21. Service: Clinical Event Syndication Stream

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-event-syndication-stream`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-CLINICAL-EVENT-SYNDICATION-STREAM`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Exchange Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-CLINICAL-EVENT-SYNDICATION-STREAM`	Direct to this identity; never reusable.
Evidence	ise:67 (p. 1466)	Source standing does not approve the derived assertion.

24.10.21.1. Definition / governed semantic detail

Emits publish-subscribe clinical event feeds.

24.10.21.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Event-Driven Syndication (p. 693)	Established

24.11. BEC CT2: Health Information Access

Property	Value	Meaning
Reference Id	`HCM-BEC-health-information-access`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	enabling:567 (p. 1379); enabling:621 (p. 1380); ise:73 (p. 1467)	Source standing does not approve the derived assertion.

24.11.1. Capability Overview

Health Information Access enables authorised discovery and retrieval across clinical information. It combines semantic search and discrete retrieval with access-qualified filtering so that a discoverable item is not automatically disclosable. It consumes established policy and consent determinations rather than making the search interface an alternative security authority.

24.11.2. Definition — Reviewed candidate

Health Information Access enables authorised discovery and retrieval across clinical information. It combines semantic search and discrete retrieval with access-qualified filtering so that a discoverable item is not automatically disclosable. It consumes established policy and consent determinations rather than making the search interface an alternative security authority.

24.11.3. Boundary — retained v0.1 source explanation

Retrieval alone does not realise the filtering Feature. Unestablished Feature associations remain unestablished even where the behaviour shares the same owner.

24.11.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Responsive governed access is a direct platform responsibility. The approved support to Individual Care Delivery remains bounded and does not establish runtime dependencies for every clinical ability.

24.11.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.11.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.11.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.11.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Search Health Information (p. 706)	function	Canonical / Established

Element / page	Type / tier	Element / placement status
Service: Clinical Information Search Service (p. 707)	service	Canonical / Established
Function: Retrieve Health Information (p. 708)	function	Canonical / Established
Service: Clinical Resource Retrieval Service (p. 709)	service	Canonical / Established
Function: Apply Access-Qualified Result Filtering (p. 709)	function	Canonical / Established
BEC CT3: Governed Health Information Retrieval (p. 703)	capability CT3	Proposed / Proposed

24.11.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT1: Individual Care Delivery (p. 5)	Established
From: enabled by	BC CT2: Individual Health Assessment (p. 6)	Established
From: enabled by	BC CT2: Clinical Determination (p. 10)	Established
From: enabled by	BC CT2: Individual Care Planning (p. 14)	Established
From: enabled by	BC CT2: Clinical Intervention (p. 18)	Established
From: enabled by	BC CT2: Individual Health Monitoring (p. 19)	Established
From: enabled by	BC CT2: Individual Care Review (p. 22)	Established
From: enabled by	BC CT2: Population Health Surveillance (p. 94)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed

24.11.10. BEC CT3: Governed Health Information Retrieval

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-governed-health-information-retrieval`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.CT3-BEC-GOVERNED-HEALTH-INFORMATION-RETRIEVAL`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.11.10.1. Capability Overview

Retrieve and search health information and apply access-qualified result filtering under established access and consent determinations. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.11.10.2. Definition — Reviewed candidate

Retrieve and search health information and apply access-qualified result filtering under established access and consent determinations.

24.11.10.3. Intent

Retrieve and search health information and apply access-qualified result filtering under established access and consent determinations.

24.11.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.11.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.11.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-73452901` (line 8848). Working locators are not identity aliases.

24.11.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Federated Clinical Query (p. 704)	feature FT	Canonical / Proposed
Feature: Access-Controlled Information Filtering (p. 705)	feature FT	Canonical / Proposed

24.11.10.8. Feature: Federated Clinical Query

Property	Value	Meaning
Reference Id	`FEAT-ISE-08`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.CT3-BEC-GOVERNED-HEALTH-INFORMATION-RETRIEVAL.FEAT-ISE-08`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access → BEC CT3: Governed Health Information Retrieval	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FEAT-ISE-08`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Access	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:623 (p. 1380)	Source standing does not approve the derived assertion.

24.11.10.8.1. Feature Overview

Federated query makes relevant clinical and directory information discoverable across distributed contexts using bounded criteria. Discovery is a governed information-access responsibility rather than authority to disclose every match or reinterpret the clinical meaning of the results.

24.11.10.8.2. Definition / governed semantic detail

Execute bounded, indexed queries across distributed clinical resources and directory graphs.

24.11.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.11.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Search Health Information (p. 706)	Established
From: associated feature	Service: Clinical Information Search Service (p. 707)	Established

24.11.10.9. Feature: Access-Controlled Information Filtering

Property	Value	Meaning
Reference Id	`FEAT-ISE-09`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.CT3-BEC-GOVERNED-HEALTH-INFORMATION-RETRIEVAL.FEAT-ISE-09`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access → BEC CT3: Governed Health Information Retrieval	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FEAT-ISE-09`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Access	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:624 (p. 1380)	Source standing does not approve the derived assertion.

24.11.10.9.1. Feature Overview

Access-qualified filtering restricts the disclosed result to the verified caller's authority and applicable consent. The responsibility is more specific than retrieving a resource; the latter's shared owner and subject matter do not establish a Feature realisation relationship.

24.11.10.9.2. Definition / governed semantic detail

Filter search results dynamically according to requesting user authorizations and patient consent directives.

24.11.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.11.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Retrieve Health Information (p. 708)	Unresolved association
From: associated feature	Service: Clinical Resource Retrieval Service (p. 709)	Unresolved association

24.11.11. Function: Search Health Information

Property	Value	Meaning
Reference Id	`HCM-FN-search-health-information`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-SEARCH-HEALTH-INFORMATION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FN-SEARCH-HEALTH-INFORMATION`	Direct to this identity; never reusable.
Evidence	ise:78 (p. 1467)	Source standing does not approve the derived assertion.

24.11.11.1. Definition / governed semantic detail

Executes structured semantic searches across diagnostic reports, clinical notes, and medication histories.

24.11.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Federated Clinical Query (p. 704)	Established

24.11.12. Service: Clinical Information Search Service

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-information-search-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-INFORMATION-SEARCH-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-INFORMATION-SEARCH-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:79 (p. 1467)	Source standing does not approve the derived assertion.

24.11.12.1. Definition / governed semantic detail

Discloses search capabilities to clinical user interfaces.

24.11.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Federated Clinical Query (p. 704)	Established

24.11.13. Function: Retrieve Health Information

Property	Value	Meaning
Reference Id	`HCM-FN-retrieve-health-information`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-RETRIEVE-HEALTH-INFORMATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FN-RETRIEVE-HEALTH-INFORMATION`	Direct to this identity; never reusable.
Evidence	ise:82 (p. 1467); ise:81 (p. 1467)	Source standing does not approve the derived assertion.

24.11.13.1. Function Overview

Retrieval obtains discrete clinical information or attachments under the owning access responsibility. It is distinct from applying the access-qualified filtering required by FEAT-ISE-09. A shared owner and related data do not justify making that unresolved realisation association authoritative.

24.11.13.2. Definition / governed semantic detail

Fetches discrete clinical observations and original attachments.

Feature association not established: `FEAT-ISE-09 — Access-Controlled Information Filtering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.

24.11.13.3. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Access-Controlled Information Filtering (p. 705)	Unresolved association

24.11.14. Service: Clinical Resource Retrieval Service

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-resource-retrieval-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-RESOURCE-RETRIEVAL-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-RESOURCE-RETRIEVAL-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:83 (p. 1467); ise:81 (p. 1467)	Source standing does not approve the derived assertion.

24.11.14.1. Definition / governed semantic detail

Delivers structured clinical information to requesting applications.

Feature association not established: `FEAT-ISE-09 — Access-Controlled Information Filtering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.

24.11.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Access-Controlled Information Filtering (p. 705)	Unresolved association

24.11.15. Function: Apply Access-Qualified Result Filtering

Property	Value	Meaning
Reference Id	`HCM-FN-apply-access-qualified-result-filtering`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-APPLY-ACCESS-QUALIFIED-RESULT-FILTERING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Access	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FN-APPLY-ACCESS-QUALIFIED-RESULT-FILTERING`	Direct to this identity; never reusable.
Evidence	ise:85 (p. 1467); ise:84 (p. 1467)	Source standing does not approve the derived assertion.

24.11.15.1. Function Overview

Result filtering changes what can be disclosed according to the verified context and applicable consent determinations. It applies established control outcomes without owning their policy. Its capability-scoped standing is retained instead of promoting the behaviour to a Feature simply to obtain traceability symmetry.

24.11.15.2. Definition / governed semantic detail

Redacts or masks search results and clinical content based on the caller's verified security context and patient consent directives.

24.12. BEC CT2: Health Information Communication

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-health-information-communication`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-COMMUNICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.12.1. Capability Overview

Enable patient notifications, patient task communication, patient self-service and access to a patient health library under applicable information-use controls. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.12.2. Definition — Reviewed candidate

Enable patient notifications, patient task communication, patient self-service and access to a patient health library under applicable information-use controls.

24.12.3. Intent

Enable patient notifications, patient task communication, patient self-service and access to a patient health library under applicable information-use controls.

24.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.12.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Patient Notification Services (p. 711)	capability CT3	Proposed / Proposed
BEC CT3: Patient Task Communication Services (p. 712)	capability CT3	Proposed / Proposed
BEC CT3: Patient Self-Service (p. 713)	capability CT3	Proposed / Proposed
BEC CT3: Patient Health Library Access (p. 714)	capability CT3	Proposed / Proposed

24.12.7. BEC CT3: Patient Notification Services

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-notification-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-COMMUNICATION.CT3-BEC-PATIENT-NOTIFICATION-SERVICES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.12.7.1. Capability Overview

Enable governed patient notifications concerning health information and applicable care or operational events. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.12.7.2. Definition — Reviewed candidate

Enable governed patient notifications concerning health information and applicable care or operational events.

24.12.7.3. Intent

Enable governed patient notifications concerning health information and applicable care or operational events.

24.12.7.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.12.7.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.12.8. BEC CT3: Patient Task Communication Services

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-task-communication-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-COMMUNICATION.CT3-BEC-PATIENT-TASK-COMMUNICATION-SERVICES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.12.8.1. Capability Overview

Communicate patient-facing work requests and their reported progression under the applicable work and disclosure contracts. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.12.8.2. Definition — Reviewed candidate

Communicate patient-facing work requests and their reported progression under the applicable work and disclosure contracts.

24.12.8.3. Intent

Communicate patient-facing work requests and their reported progression under the applicable work and disclosure contracts.

24.12.8.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.12.8.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.12.9. BEC CT3: Patient Self-Service

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-self-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-COMMUNICATION.CT3-BEC-PATIENT-SELF-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.12.9.1. Capability Overview

Enable patients to undertake permitted information-access and administrative self-service interactions. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.12.9.2. Definition — Reviewed candidate

Enable patients to undertake permitted information-access and administrative self-service interactions.

24.12.9.3. Intent

Enable patients to undertake permitted information-access and administrative self-service interactions.

24.12.9.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.12.9.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.12.10. BEC CT3: Patient Health Library Access

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-health-library-access`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-COMMUNICATION.CT3-BEC-PATIENT-HEALTH-LIBRARY-ACCESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.12.10.1. Capability Overview

Enable authorised patient access to appropriately governed health knowledge and library information. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.12.10.2. Definition — Reviewed candidate

Enable authorised patient access to appropriately governed health knowledge and library information.

24.12.10.3. Intent

Enable authorised patient access to appropriately governed health knowledge and library information.

24.12.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.12.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.13. BEC CT2: Health Information Control

Property	Value	Meaning
Reference Id	`HCM-BEC-health-information-control`	Stable concept identity.

Property	Value	Meaning
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:569 (p. 1379); enabling:630 (p. 1380); ise:104 (p. 1468)	Source standing does not approve the derived assertion.

24.13.1. Consent responsibility

BEC CT3: Client Privacy Guardrail Management retains client consent directives and FEAT-EM-09. This branch respects that responsibility without duplicating it.

24.13.2. Capability Overview

Govern patient health-information restrictions, filtering and disclosure using established information-access authority, client privacy/consent determinations and applicable security controls.

24.13.3. Definition — Reviewed candidate

Govern patient health-information restrictions, filtering and disclosure using established information-access authority, client privacy/consent determinations and applicable security controls.

24.13.4. Boundary — retained v0.1 source explanation

Security context is operational context and does not automatically become resource content. Security-context validation has no invented one-to-one mapping to propagation, and audit recording is not an assurance conclusion.

24.13.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Assigned policy enforcement and attributable context are substantive Core responsibilities across managed operations.

24.13.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity. Client consent directives remain owned by Client Privacy Guardrail Management (FEAT-EM-09); this capability references and respects those determinations without defining a second consent owner.

24.13.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.13.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.13.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Evaluate Information Access Authority (p. 722)	function	Canonical / Established
Service: Policy Evaluation & Authorisation Service (p. 723)	service	Canonical / Established
Function: Evaluate Consent Constraint (p. 724)	function	Canonical / Established
Service: Consent Enforcement Service (p. 724)	service	Canonical / Established
Function: Propagate Security Context (p. 725)	function	Canonical / Established
Service: Security Context Validation Service (p. 726)	service	Canonical / Established
Function: Record Security-Significant Activity (p. 727)	function	Canonical / Established
Service: Security Audit Ingress (p. 727)	service	Canonical / Established
BEC CT3: Patient Health Information Restriction Management (p. 718)	capability CT3	Proposed / Proposed
BEC CT3: Patient Health Information Filter Management (p. 720)	capability CT3	Proposed / Proposed
BEC CT3: Patient Health Information Disclosure Management (p. 721)	capability CT3	Proposed / Proposed

24.13.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Digital Identity & Access Governance (p. 141)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed
To: enterprise enabled by	Enterprise CT2: Context Management (p. 839)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed
To: enterprise enabled by	Enterprise CT3: Governed Activity Audit Evidence (p. 872)	Proposed

24.13.11. BEC CT3: Patient Health Information Restriction Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-health-information-restriction-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.CT3-BEC-PATIENT-HEALTH-INFORMATION-RESTRICTION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.13.11.1. Consent responsibility

BEC CT3: Client Privacy Guardrail Management retains client consent directives and FEAT-EM-09. This branch respects that responsibility without duplicating it.

24.13.11.2. Capability Overview

Manage application of patient health-information restrictions using the client-associated privacy and consent responsibilities retained by Client Privacy Guardrail Management. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.13.11.3. Definition — Reviewed candidate

Manage application of patient health-information restrictions using the client-associated privacy and consent responsibilities retained by Client Privacy Guardrail Management.

24.13.11.4. Intent

Manage application of patient health-information restrictions using the client-associated privacy and consent responsibilities retained by Client Privacy Guardrail Management.

24.13.11.5. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

Client consent directives remain owned by Client Privacy Guardrail Management (FEAT-EM-09); this capability references and respects those determinations without defining a second consent owner.

24.13.11.6. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.13.11.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Health Information Access & Consent Control (p. 719)	feature FT	Canonical / Proposed

24.13.11.8. Feature: Health Information Access & Consent Control

Property	Value	Meaning
Reference Id	`FEAT-ISE-11`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.CT3-BEC-PATIENT-HEALTH-INFORMATION-RESTRICTION-MANAGEMENT.FEAT-ISE-11`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control → BEC CT3: Patient Health Information Restriction Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-ISE-11`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Health Information Control	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:632 (p. 1381)	Source standing does not approve the derived assertion.

24.13.11.8.1. Feature Overview

Access and consent control evaluates whether an information operation is legitimate in its practitioner, patient and purpose context. It supplies policy determinations consumed by access and presentation paths rather than allowing those consumers to establish authority from caller-supplied content.

24.13.11.8.2. Definition / governed semantic detail

Evaluate patient consent preferences, practitioner authority boundaries, and access control policies for managed clinical records.

24.13.11.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.13.11.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Evaluate Information Access Authority (p. 722)	Established
From: associated feature	Service: Policy Evaluation & Authorisation Service (p. 723)	Established

24.13.12. BEC CT3: Patient Health Information Filter Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-health-information-filter-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.CT3-BEC-PATIENT-HEALTH-INFORMATION-FILTER-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.13.12.1. Consent responsibility

BEC CT3: Client Privacy Guardrail Management retains client consent directives and FEAT-EM-09. This branch respects that responsibility without duplicating it.

24.13.12.2. Capability Overview

Manage patient health-information filtering rules and qualified projections while respecting established client privacy and consent determinations. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.13.12.3. Definition — Reviewed candidate

Manage patient health-information filtering rules and qualified projections while respecting established client privacy and consent determinations.

24.13.12.4. Intent

Manage patient health-information filtering rules and qualified projections while respecting established client privacy and consent determinations.

24.13.12.5. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority. Client consent directives remain owned by Client Privacy Guardrail Management (FEAT-EM-09); this capability references and respects those determinations without defining a second consent owner.

24.13.12.6. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.13.13. BEC CT3: Patient Health Information Disclosure Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-patient-health-information-disclosure-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.CT3-BEC-PATIENT-HEALTH-INFORMATION-DISCLOSURE-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.13.13.1. Consent responsibility

BEC CT3: Client Privacy Guardrail Management retains client consent directives and FEAT-EM-09. This branch respects that responsibility without duplicating it.

24.13.13.2. Capability Overview

Manage permitted patient health-information disclosure and its applicable recipient, purpose and restriction conditions. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.13.13.3. Definition — Reviewed candidate

Manage permitted patient health-information disclosure and its applicable recipient, purpose and restriction conditions.

24.13.13.4. Intent

Manage permitted patient health-information disclosure and its applicable recipient, purpose and restriction conditions.

24.13.13.5. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority. Client consent directives remain owned by Client Privacy Guardrail Management (FEAT-EM-09); this capability references and respects those determinations without defining a second consent owner.

24.13.13.6. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.13.14. Function: Evaluate Information Access Authority

Property	Value	Meaning
Reference Id	`HCM-FN-evaluate-information-access-authority`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-INFORMATION-ACCESS-AUTHORITY`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-INFORMATION-ACCESS-AUTHORITY`	Direct to this identity; never reusable.
Evidence	ise:108 (p. 1468)	Source standing does not approve the derived assertion.

24.13.14.1. Definition / governed semantic detail

Evaluates user role, organisation, patient relationship, purpose of use, and break-glass overrides against default-deny policies.

24.13.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Health Information Access & Consent Control (p. 719)	Established

24.13.15. Service: Policy Evaluation & Authorisation Service

Property	Value	Meaning
Reference Id	`HCM-SV-policy-evaluation-authorisation-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-POLICY-EVALUATION-AUTHORISATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-POLICY-EVALUATION-AUTHORISATION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:109 (p. 1468)	Source standing does not approve the derived assertion.

24.13.15.1. Definition / governed semantic detail

Evaluates access requests across the platform.

24.13.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Health Information Access & Consent Control (p. 719)	Established

24.13.16. Function: Evaluate Consent Constraint

Property	Value	Meaning
Reference Id	`HCM-FN-evaluate-consent-constraint`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-CONSENT-CONSTRAINT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-CONSENT-CONSTRAINT`	Direct to this identity; never reusable.
Evidence	ise:112 (p. 1468); ise:111 (p. 1468)	Source standing does not approve the derived assertion.

24.13.16.1. Definition / governed semantic detail

Cross-references access requests against client consent directives and sensitive health category restrictions.

Feature association not established: `FEAT-ISE-12 — Security Context Propagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.

24.13.17. Service: Consent Enforcement Service

Property	Value	Meaning
Reference Id	`HCM-SV-consent-enforcement-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-CONSENT-ENFORCEMENT-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-CONSENT-ENFORCEMENT-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:113 (p. 1468); ise:111 (p. 1468)	Source standing does not approve the derived assertion.

24.13.17.1. Definition / governed semantic detail

Returns masking and redaction instructions.

Feature association not established: `FEAT-ISE-12 — Security Context Propagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.

24.13.18. Function: Propagate Security Context

Property	Value	Meaning
Reference Id	`HCM-FN-propagate-security-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-PROPAGATE-SECURITY-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-PROPAGATE-SECURITY-CONTEXT`	Direct to this identity; never reusable.
Evidence	ise:115 (p. 1468); ise:114 (p. 1468); ise:117 (p. 1468)	Source standing does not approve the derived assertion.

24.13.18.1. Function Overview

Propagation preserves attributable operational security context across managed transactions. It provides continuity of context rather than independently deciding permission for every down-

stream action. The nearby validation Service has its own declared exposure; juxtaposition does not establish an exact one-to-one implementation relationship.

24.13.18.2. Definition / governed semantic detail

Generates and binds canonical, immutable security contexts to all intra-platform transactions.

Exposure qualification: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.

24.13.19. Service: Security Context Validation Service

Property	Value	Meaning
Reference Id	`HCM-SV-security-context-validation-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-CONTEXT-VALIDATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-CONTEXT-VALIDATION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:116 (p. 1468); ise:114 (p. 1468); ise:117 (p. 1468)	Source standing does not approve the derived assertion.

24.13.19.1. Service Overview

This Service lets a client obtain validation of transaction security credentials. The exposure is established even though its precise internal Function mapping is not. Validation must not be rewritten as context propagation merely to manufacture a symmetrical Function-to-Service pair.

24.13.19.2. Definition / governed semantic detail

Verifies transaction security credentials.

Exposure qualification: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.

24.13.20. Function: Record Security-Significant Activity

Property	Value	Meaning
Reference Id	`HCM-FN-record-security-significant-activity`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-RECORD-SECURITY-SIGNIFICANT-ACTIVITY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-RECORD-SECURITY-SIGNIFICANT-ACTIVITY`	Direct to this identity; never reusable.
Evidence	ise:119 (p. 1468)	Source standing does not approve the derived assertion.

24.13.20.1. Definition / governed semantic detail

Records tamper-evident audit evidence (who, what, when, why, patient MRN, policy outcome) without logging unmasked PHI.

24.13.21. Service: Security Audit Ingress

Property	Value	Meaning
Reference Id	`HCM-SV-security-audit-ingress`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-AUDIT-INGRESS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-AUDIT-INGRESS`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	ise:120 (p. 1468)	Source standing does not approve the derived assertion.

24.13.21.1. Definition / governed semantic detail

Ingests audit events across all capabilities.

24.14. BEC CT2: Information Asset Stewardship Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-information-asset-stewardship-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-ASSET-STEWARDSHIP-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.14.1. Capability Overview

Manage definitions and tags that make governed information assets recognisable and interpretable for their permitted uses. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.14.2. Definition — Reviewed candidate

Manage definitions and tags that make governed information assets recognisable and interpretable for their permitted uses.

24.14.3. Intent

Manage definitions and tags that make governed information assets recognisable and interpretable for their permitted uses.

24.14.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.14.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.14.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Information Asset Definition (p. 729)	capability CT3	Proposed / Proposed
BEC CT3: Information Asset Tag Management (p. 730)	capability CT3	Proposed / Proposed

24.14.7. BEC CT3: Information Asset Definition

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-information-asset-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-ASSET-STEWARDSHIP-MANAGEMENT.CT3-BEC-INFORMATION-ASSET-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Asset Stewardship Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.14.7.1. Capability Overview

Define the identifying, semantic and governed-use characteristics of an information asset. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.14.7.2. Definition — Reviewed candidate

Define the identifying, semantic and governed-use characteristics of an information asset.

24.14.7.3. Intent

Define the identifying, semantic and governed-use characteristics of an information asset.

24.14.7.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.14.7.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.14.8. BEC CT3: Information Asset Tag Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-information-asset-tag-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-ASSET-STEWARDSHIP-MANAGEMENT.CT3-BEC-INFORMATION-ASSET-TAG-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Asset Stewardship Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.14.8.1. Capability Overview

Manage attributable tags describing information assets without treating caller-controlled tags as security authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.14.8.2. Definition — Reviewed candidate

Manage attributable tags describing information assets without treating caller-controlled tags as security authority.

24.14.8.3. Intent

Manage attributable tags describing information assets without treating caller-controlled tags as security authority.

24.14.8.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.14.8.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.15. BEC CT2: Information Content Maintainer Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-information-content-maintainer-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.15.1. Capability Overview

Administer information-content maintainer participation, source provenance and governed information products. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.15.2. Definition — Reviewed candidate

Administer information-content maintainer participation, source provenance and governed information products.

24.15.3. Intent

Administer information-content maintainer participation, source provenance and governed information products.

24.15.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.15.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.15.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Data Provenance Capture (p. 733)	capability CT3	Proposed / Proposed
BEC CT3: Data Scientist Administration (p. 732)	capability CT3	Proposed / Proposed
BEC CT3: Information Product Administration (p. 737)	capability CT3	Proposed / Proposed

24.15.7. BEC CT3: Data Scientist Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-data-scientist-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-DATA-SCIENTIST-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.15.7.1. Capability Overview

Administer profiles and qualified participation information for data scientists maintaining governed information content. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.15.7.2. Definition — Reviewed candidate

Administer profiles and qualified participation information for data scientists maintaining governed information content.

24.15.7.3. Intent

Administer profiles and qualified participation information for data scientists maintaining governed information content.

24.15.7.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.15.7.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.15.8. BEC CT3: Data Provenance Capture

Property	Value	Meaning
Reference Id	`HCM-BEC-information-lineage-stewardship`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP`	Direct to this identity; never reusable.
Name history	Information Lineage Stewardship	Names do not establish another identity.

Property	Value	Meaning
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	architectural-axioms.md:411; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.15.8.1. Capability Overview

Capture meaningful source attribution and transformation lineage for governed information content without transferring originating authority.

24.15.8.2. Definition — Reviewed candidate

Capture meaningful source attribution and transformation lineage for governed information content without transferring originating authority.

24.15.8.3. Boundary

Lineage does not grant originating authority, decide clinical correctness or replace activity audit and independent assurance.

24.15.8.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.15.8.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

24.15.8.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:411; information-responsibility.md:10

24.15.8.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.15.8.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Maintain source attribution and meaningful transformation lineage for health information managed across care boundaries.

Previous overview: Recipients need to know which assertions were received, transformed or superseded and why. This ability preserves those meaningful relationships without making every retry or active-state reconstruction permanent provenance.

Previous intent: Allow a reader to explain the meaningful source and transformation history of a managed assertion.

24.15.8.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Information Transformation Lineage (p. 735)	feature FT	Proposed / Proposed
Function: Record Significant Information Lineage (p. 736)	function	Proposed / Proposed

24.15.8.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Digital Accountability & Trust (p. 155)	Proposed
To: enterprise enabled by	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed

24.15.8.11. Feature: Information Transformation Lineage

Property	Value	Meaning
Reference Id	`HCM-FT-information-transformation-lineage`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FEAT-STEP2-INFORMATION-TRANSFORMATION-LINEAGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management → BEC CT3: Data Provenance Capture	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FEAT-STEP2-INFORMATION-TRANSFORMATION-LINEAGE`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

24.15.8.11.1. Feature Overview

A consumer should be able to explain which source assertions contributed to a managed representation and what significant transformation occurred.

24.15.8.11.2. Definition — Proposed

Record meaningful source-to-derived information relationships and transformation attribution.

24.15.8.11.3. Boundary

Routine cache reconstruction, retries and diagnostics do not automatically become permanent lineage.

24.15.8.11.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Information Lineage Stewardship, rather than inheriting an adjacent responsibility.

24.15.8.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

24.15.8.11.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-3149DD0E` (line 8678). Working locators are not identity aliases.

24.15.8.11.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Record Significant Information Lineage (p. 736)	Proposed

24.15.8.12. Function: Record Significant Information Lineage

Property	Value	Meaning
Reference Id	`HCM-FN-record-significant-information-lineage`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FN-RECORD-SIGNIFICANT-INFORMATION-LINEAGE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management → BEC CT3: Data Provenance Capture	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FN-RECORD-SIGNIFICANT-INFORMATION-LINEAGE`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

24.15.8.12.1. Function Overview

Changes an unexplained derived representation into an attributable relationship to the source information and transformation basis.

24.15.8.12.2. Definition — Proposed

Associate a managed assertion with its attributed sources and information-significant transformation.

24.15.8.12.3. Boundary

Routine retries, cache mechanics and debug traces are excluded unless information-significant.

24.15.8.12.4. Proposed Placement Rationale

Behaviour remains within Information Lineage Stewardship responsibility.

24.15.8.12.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

24.15.8.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Information Transformation Lineage (p. 735)	Proposed

24.15.9. BEC CT3: Information Product Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-information-product-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.15.9.1. Capability Overview

Administer identifiable governed information products, their purpose, content standing and qualified release/use context. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.15.9.2. Definition — Reviewed candidate

Administer identifiable governed information products, their purpose, content standing and qualified release/use context.

24.15.9.3. Intent

Administer identifiable governed information products, their purpose, content standing and qualified release/use context.

24.15.9.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.15.9.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.15.9.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Authorised Research Release Assembly (p. 739)	feature FT	Proposed / Proposed
Feature: Population Notification Obligation Tracking (p. 740)	feature FT	Proposed / Proposed
Feature: Demand & Capacity Planning Projection (p. 741)	feature FT	Proposed / Proposed

24.15.9.7. Feature: Authorised Research Release Assembly

Property	Value	Meaning
Reference Id	`HCM-FT-authorised-research-release-assembly`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-AUTHORISED-RESEARCH-RELEASE-ASSEMBLY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management → BEC CT3: Information Product Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FEAT-STEP2-AUTHORISED-RESEARCH-RELEASE-ASSEMBLY`	Direct to this identity; never reusable.
Evidence	business-capabilities.md:842; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.15.9.7.1. Purpose context

BEC CT2: Research Information Curation (p. 812). Shared product framing preserves these existing purpose-specific responsibilities without inventing CT3 decomposition beneath each purpose.

24.15.9.7.2. Feature Overview

The release carries purpose and applicable privacy/consent/use constraints; the requester does not gain unrestricted clinical-record access by having a research role.

24.15.9.7.3. Definition — Proposed

Assemble and qualify the existing information allowed by a particular authorised research purpose.

24.15.9.7.4. Boundary

No ethics approval, de-identification guarantee or clinical-trial management is inferred.

24.15.9.7.5. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.15.9.7.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:842; information-responsibility.md:10

24.15.9.7.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Assemble Purpose-Qualified Research Release (p. 814)	Proposed
To: purpose context	BEC CT2: Research Information Curation (p. 812)	Proposed

24.15.9.8. Feature: Population Notification Obligation Tracking

Property	Value	Meaning
Reference Id	`HCM-FT-population-notification-obligation-tracking`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-POPULATION-NOTIFICATION-OBLIGATION-TRACKING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management → BEC CT3: Information Product Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT.FEAT-STEP2-POPULATION-NOTIFICATION-OBLIGATION-TRACKING`	Direct to this identity; never reusable.
Evidence	business-capabilities.md:611	Source standing does not approve the derived assertion.

24.15.9.8.1. Purpose context

BEC CT2: Population Health Information Reporting (p. 817). Shared product framing preserves these existing purpose-specific responsibilities without inventing CT3 decomposition beneath each purpose.

24.15.9.8.2. Feature Overview

Sending a required notification and discharging the governing obligation are different assertions. This Feature keeps the explicit contract and indeterminate responses visible.

24.15.9.8.3. Definition — Proposed

Track the relation between an applicable notification obligation, the supplied source evidence and observed notification disposition.

24.15.9.8.4. Boundary

Does not define notifiability, author statutory rules or own public-health response.

24.15.9.8.5. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.15.9.8.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:611

24.15.9.8.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: purpose context	BEC CT2: Population Health Information Reporting (p. 817)	Proposed

24.15.9.9. Feature: Demand & Capacity Planning Projection

Property	Value	Meaning
Reference Id	`HCM-FT-demand-capacity-planning-projection`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-DEMAND-CAPACITY-PLANNING-PROJECTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Content Maintainer Management → BEC CT3: Information Product Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-SERVICE-PLANNING-INFORMATION-PROVISION.FEAT-STEP2-DEMAND-CAPACITY-PLANNING-PROJECTION`	Direct to this identity; never reusable.
Evidence	business-capabilities.md:626; information-responsibility.md:34	Source standing does not approve the derived assertion.

24.15.9.9.1. Purpose context

BEC CT2: Service Planning Data Management (p. 818). Shared product framing preserves these existing purpose-specific responsibilities without inventing CT3 decomposition beneath each purpose.

24.15.9.9.2. Feature Overview

Comparisons need service, temporal and context qualifiers so that planned future capacity is not confused with present operational capacity.

24.15.9.9.3. Definition — Proposed

Project qualified observed demand, capacity and utilisation for authorised planning consumers.

24.15.9.9.4. Boundary

Projection does not establish a planning conclusion or allocate services and staff.

24.15.9.9.5. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.15.9.9.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:626; information-responsibility.md:34

24.15.9.9.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: purpose context	BEC CT2: Service Planning Data Management (p. 818)	Proposed

24.16. BEC CT2: Clinical Knowledge Services

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-knowledge-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:570 (p. 1379); enabling:636 (p. 1381); ise:126 (p. 1468)	Source standing does not approve the derived assertion.

24.16.1. Capability Overview

Administer clinical knowledge requests, distribute governed knowledge artefacts and administer approved reference datasets.

24.16.2. Definition — Reviewed candidate

Administer clinical knowledge requests, distribute governed knowledge artefacts and administer approved reference datasets.

24.16.3. Boundary — retained v0.1 source explanation

Code translation does not establish equivalence beyond the governed map or confer clinical decision authority.

24.16.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Concept resolution and mapping materially support meaningful information exchange and interpretation across regional systems.

24.16.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.16.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.16.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.16.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Resolve Clinical Concept (p. 753)	function	Canonical / Established
Service: Terminology Concept Resolution Service (p. 754)	service	Canonical / Established
Function: Map Clinical Concept (p. 755)	function	Canonical / Established
Service: Terminology Mapping Service (p. 755)	service	Canonical / Established
BEC CT3: Governed Reference Dataset Administration (p. 748)	capability CT3	Proposed / Proposed
BEC CT3: Clinical Knowledge Request Administration (p. 744)	capability CT3	Proposed / Proposed
BEC CT3: Clinical Knowledge Artefact Distribution Management (p. 747)	capability CT3	Proposed / Proposed

24.16.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Clinical Knowledge Management (p. 112)	Proposed
From: enabled by	BC CT2: Clinical Semantics Governance (p. 128)	Proposed
To: enterprise enabled by	Enterprise CT2: Search & Discovery (p. 849)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed
To: enterprise enabled by	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

24.16.10. BEC CT3: Clinical Knowledge Request Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-knowledge-request-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.16.10.1. Capability Overview

Administer clinical knowledge requests and their qualified resolution to applicable governed knowledge or reference content. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.16.10.2. Definition — Reviewed candidate

Administer clinical knowledge requests and their qualified resolution to applicable governed knowledge or reference content.

24.16.10.3. Intent

Administer clinical knowledge requests and their qualified resolution to applicable governed knowledge or reference content.

24.16.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.16.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.16.10.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Clinical Concept Resolution (p. 745)	feature FT	Canonical / Proposed
Feature: Bidirectional Terminology Mapping (p. 746)	feature FT	Canonical / Proposed

24.16.10.7. Feature: Clinical Concept Resolution

Property	Value	Meaning
Reference Id	`FEAT-ISE-14`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-14`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services → BEC CT3: Clinical Knowledge Request Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-ISE-14`; `CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FEAT-ISE-14`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Knowledge Services	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:638 (p. 1381)	Source standing does not approve the derived assertion.

24.16.10.7.1. Feature Overview

Concept resolution makes a coded clinical reference interpretable against its governed terminology. It validates the reference context without making a recognised code proof of clinical truth or giving the consuming system authority to redefine the concept.

24.16.10.7.2. Definition / governed semantic detail

Resolve local clinical codes against standard terminologies (SNOMED CT, LOINC, ICD-10).

24.16.10.7.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.16.10.7.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Resolve Clinical Concept (p. 753)	Established
From: associated feature	Service: Terminology Concept Resolution Service (p. 754)	Established

24.16.10.8. Feature: Bidirectional Terminology Mapping

Property	Value	Meaning
Reference Id	FEAT-ISE-15	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-15	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services → BEC CT3: Clinical Knowledge Request Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-ISE-15; CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FEAT-ISE-15	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Knowledge Services	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:639 (p. 1381)	Source standing does not approve the derived assertion.

24.16.10.8.1. Feature Overview

Terminology mapping applies governed correspondence between local and canonical semantic systems. Translation preserves the limitations and authority of the map; it does not justify silently treating related source concepts as identical or clinically adjudicating the underlying information.

24.16.10.8.2. Definition / governed semantic detail

Translate between legacy local code systems and canonical national healthcare profiles.

24.16.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.16.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Map Clinical Concept (p. 755)	Established
From: associated feature	Service: Terminology Mapping Service (p. 755)	Established

24.16.11. BEC CT3: Clinical Knowledge Artefact Distribution Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-knowledge-artefact-distribution-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-ARTEFACT-DISTRIBUTION-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.16.11.1. Capability Overview

Manage distribution of governed clinical knowledge artefacts to intended consumers under their applicable use contracts. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.16.11.2. Definition — Reviewed candidate

Manage distribution of governed clinical knowledge artefacts to intended consumers under their applicable use contracts.

24.16.11.3. Intent

Manage distribution of governed clinical knowledge artefacts to intended consumers under their applicable use contracts.

24.16.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.16.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.16.12. BEC CT3: Governed Reference Dataset Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-governed-reference-dataset-administration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION`	Direct to this identity; never reusable.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	calliope-models-and-datasets.md:120; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.16.12.1. Capability Overview

Administer approved runtime reference-dataset content and revisions separately from the reusable models defining its meaning.

24.16.12.2. Definition — Reviewed candidate

Administer approved runtime reference-dataset content and revisions separately from the reusable models defining its meaning.

24.16.12.3. Boundary

Does not own external ontology authority, execute authorisation policy or establish durable truth independently of the normal persistence boundary.

24.16.12.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.16.12.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

24.16.12.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task calliope-models-and-datasets.md:120; information-responsibility.md:10

24.16.12.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.16.12.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Manage the approved runtime content and revision standing of health-care reference datasets.

Previous overview: Value-set membership, mapping content and governed nomenclature may change independently of the structural definition that gives them meaning. Updates need identifiable review and effective standing without redefining model libraries as mutable data.

Previous intent: Make approved reference-content change usable during operation without changing the meaning of its structural definition.

24.16.12.9. Prior Candidate Derivation

v0.3 reconciliation: `BEC-43B6CE64` (line 9123). Working locators are not identity aliases.

24.16.12.10. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Governed Dataset Content Revision (p. 750)	feature FT	Proposed / Proposed
Function: Accept Governed Dataset Revision (p. 751)	function	Proposed / Proposed
Service: Governed Reference Dataset Content Query (p. 752)	service	Proposed / Proposed

24.16.12.11. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Clinical Knowledge Management (p. 112)	Proposed
To: enterprise enabled by	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed
To: enterprise enabled by	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed
To: enterprise enabled by	Enterprise CT3: Active Information Availability (p. 846)	Proposed
To: enterprise enabled by	Enterprise CT3: Authoritative Durable State Establishment (p. 847)	Proposed

24.16.12.12. Feature: Governed Dataset Content Revision

Property	Value	Meaning
Reference Id	`HCM-FT-governed-dataset-content-revision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-STEP2-GOVERNED-DATASET-CONTENT-REVISION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services → BEC CT3: Governed Reference Dataset Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-STEP2-GOVERNED-DATASET-CONTENT-REVISION`	Direct to this identity; never reusable.
Evidence	calliope-models-and-datasets.md:120	Source standing does not approve the derived assertion.

24.16.12.12.1. Feature Overview

A model definition can remain stable while approved terminology membership or mapping content changes. This Feature represents that operational governance without making every definition runtime mutable.

24.16.12.12.2. Definition — Proposed

Review, accept and expose authorised changes to mutable reference content with effective revision standing.

24.16.12.12.3. Boundary

Does not invent an approval authority or bypass normal durable establishment.

24.16.12.12.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Governed Reference Dataset Administration, rather than inheriting an adjacent responsibility.

24.16.12.12.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task calliope-models-and-datasets.md:120

24.16.12.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Accept Governed Dataset Revision (p. 751)	Proposed

24.16.12.13. Function: Accept Governed Dataset Revision

Property	Value	Meaning
Reference Id	`HCM-FN-accept-governed-dataset-revision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FN-ACCEPT-GOVERNED-DATASET-REVISION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services → BEC CT3: Governed Reference Dataset Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FN-ACCEPT-GOVERNED-DATASET-REVISION`	Direct to this identity; never reusable.
Evidence	calliope-models-and-datasets.md:120	Source standing does not approve the derived assertion.

24.16.12.13.1. Function Overview

Moves a reviewed change to the applicable approved content revision, retaining attribution and uncertainty if durable establishment cannot be confirmed.

24.16.12.13.2. Definition — Proposed

Apply an explicitly authorised content revision and its effective standing through governed information-state progression.

24.16.12.13.3. Boundary

The Function does not invent who approves or become a persistence manager.

24.16.12.13.4. Proposed Placement Rationale

Behaviour remains within Governed Reference Dataset Administration responsibility.

24.16.12.13.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task calliope-models-and-datasets.md:120

24.16.12.13.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Governed Dataset Content Revision (p. 750)	Proposed

24.16.12.14. Service: Governed Reference Dataset Content Query

Property	Value	Meaning
Reference Id	`HCM-SV-governed-reference-dataset-content-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.SV-GOVERNED-REFERENCE-DATASET-CONTENT-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services → BEC CT3: Governed Reference Dataset Administration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.SV-GOVERNED-REFERENCE-DATASET-CONTENT-QUERY`	Direct to this identity; never reusable.
Evidence	calliope-models-and-datasets.md:120	Source standing does not approve the derived assertion.

24.16.12.14.1. Service Overview

Consumers can resolve the membership or mapping content they actually used without treating build-time model definitions as runtime data.

24.16.12.14.2. Definition — Proposed

Provide authorised consumers with approved content at an identified revision and applicability basis.

24.16.12.14.3. Boundary

Does not grant unrestricted dataset mutation or external terminology authority.

24.16.12.14.4. Proposed Placement Rationale

Behaviour remains within Governed Reference Dataset Administration responsibility.

24.16.12.14.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task calliope-models-and-datasets.md:120

24.16.13. Function: Resolve Clinical Concept

Property	Value	Meaning
Reference Id	`HCM-FN-resolve-clinical-concept`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-RESOLVE-CLINICAL-CONCEPT`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-RESOLVE-CLINICAL-CONCEPT`	Direct to this identity; never reusable.
Evidence	ise:130 (p. 1468)	Source standing does not approve the derived assertion.

24.16.13.1. Definition / governed semantic detail

Validates concept identifiers and retrieves preferred clinical rubrics and hierarchy paths.

24.16.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Clinical Concept Resolution (p. 745)	Established

24.16.14. Service: Terminology Concept Resolution Service

Property	Value	Meaning
Reference Id	`HCM-SV-terminology-concept-resolution-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-CONCEPT-RESOLUTION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-CONCEPT-RESOLUTION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:131 (p. 1468)	Source standing does not approve the derived assertion.

24.16.14.1. Definition / governed semantic detail

Exposes concept validation lookups.

24.16.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Clinical Concept Resolution (p. 745)	Established

24.16.15. Function: Map Clinical Concept

Property	Value	Meaning
Reference Id	`HCM-FN-map-clinical-concept`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-MAP-CLINICAL-CONCEPT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-MAP-CLINICAL-CONCEPT`	Direct to this identity; never reusable.
Evidence	ise:133 (p. 1468)	Source standing does not approve the derived assertion.

24.16.15.1. Definition / governed semantic detail

Translates local lab codes and legacy terms to canonical SNOMED/LOINC codes using governed translation maps.

24.16.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Bidirectional Terminology Mapping (p. 746)	Established

24.16.16. Service: Terminology Mapping Service

Property	Value	Meaning
Reference Id	`HCM-SV-terminology-mapping-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-MAPPING-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Knowledge Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-MAPPING-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:134 (p. 1468)	Source standing does not approve the derived assertion.

24.16.16.1. Definition / governed semantic detail

Translates codes across terminology systems.

24.16.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Bidirectional Terminology Mapping (p. 746)	Established

24.17. BEC CT2: Clinical Collaboration

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	enabling:571 (p. 1379); enabling:641 (p. 1381); ise:140 (p. 1469)	Source standing does not approve the derived assertion.

24.17.1. Capability Overview

Clinical Collaboration provides secure multidisciplinary spaces bound to clinical context, including governed access to longitudinal information during discussion. Its purpose is contextual conversation between care participants rather than a separate source of authoritative clinical facts. A statement in a conversation remains discourse until explicitly authored through the governed record boundary.

24.17.2. Definition — Reviewed candidate

Clinical Collaboration provides secure multidisciplinary spaces bound to clinical context, including governed access to longitudinal information during discussion. Its purpose is contextual conversation between care participants rather than a separate source of authoritative clinical facts. A statement in a conversation remains discourse until explicitly authored through the governed record boundary.

24.17.3. Boundary — retained v0.1 source explanation

Channel metadata must remain free of PHI. The source Function's mention of managing clinical handovers conflicts with the metamodel's clinical-work boundary; both are preserved for review without assigning that authority here.

24.17.4. Harmonia Architectural Relevance — retained v0.1 source explanation

LHR-aware collaboration and controlled contextual queries are substantive Core responsibilities, distinct from clinical practice or record authorship.

24.17.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.17.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.17.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.17.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Coordinate Care-Team Collaboration (p. 763)	function	Canonical / Established
Service: Clinical Collaboration Space Service (p. 764)	service	Canonical / Established
Function: Resolve LHR Query within Collaboration (p. 765)	function	Canonical / Established
Service: Collaboration Clinical Summary Resolution (p. 766)	service	Canonical / Established
Function: Govern Collaboration Metadata (p. 766)	function	Canonical / Established
Service: Collaboration Space Governance Service (p. 767)	service	Canonical / Established
BEC CT3: Clinical Context Collaboration (p. 758)	capability CT3	Proposed / Proposed

24.17.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Care Coordination & Transition (p. 37)	Proposed
To: enterprise enabled by	Enterprise CT2: Interaction & Experience (p. 862)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed
To: enterprise enabled by	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed

24.17.10. BEC CT3: Clinical Context Collaboration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-clinical-context-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.17.10.1. Capability Overview

Enable governed multidisciplinary collaboration with clinical context while preserving the distinction between discourse and authored clinical records. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.17.10.2. Definition — Reviewed candidate

Enable governed multidisciplinary collaboration with clinical context while preserving the distinction between discourse and authored clinical records.

24.17.10.3. Intent

Enable governed multidisciplinary collaboration with clinical context while preserving the distinction between discourse and authored clinical records.

24.17.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.17.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.17.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-0C37BAFF` (line 9222). Working locators are not identity aliases.

24.17.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Care-Team Clinical Collaboration Coordination (p. 759)	feature FT	Canonical / Proposed
Feature: In-Conversation LHR Query Resolution (p. 760)	feature FT	Canonical / Proposed
Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	feature FT	Canonical / Proposed
Feature: Collaboration Interaction Capture Qualification (p. 762)	feature FT	Proposed / Proposed

24.17.10.8. Feature: Care-Team Clinical Collaboration Coordination

Property	Value	Meaning
Reference Id	`FEAT-ISE-16`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-16`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration → BEC CT3: Clinical Context Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-ISE-16`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Collaboration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:643 (p. 1381)	Source standing does not approve the derived assertion.

24.17.10.8.1. Feature Overview

Clinical collaboration coordination binds secure multidisciplinary exchange to the relevant encounter or clinical topic. It supports conversation among authorised participants without making discourse an authoritative observation or acquiring the clinical handover authority excluded by the metamodel.

24.17.10.8.2. Definition / governed semantic detail

Coordinate secure multidisciplinary clinical communication and consultation exchange bound to patient encounters or clinical topics.

24.17.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.17.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Coordinate Care-Team Collaboration (p. 763)	Established
From: associated feature	Service: Clinical Collaboration Space Service (p. 764)	Established

24.17.10.9. Feature: In-Conversation LHR Query Resolution

Property	Value	Meaning
Reference Id	`FEAT-ISE-17`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-17`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration → BEC CT3: Clinical Context Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-ISE-17`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Collaboration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:644 (p. 1381)	Source standing does not approve the derived assertion.

24.17.10.9.1. Feature Overview

In-conversation query supplies governed clinical summaries within an authorised collaboration context. The convenience of retrieving information during discussion does not weaken consent or access controls, and displaying a summary does not promote the surrounding conversation into the clinical record.

24.17.10.9.2. Definition / governed semantic detail

Resolve clinical data queries within an authorized collaboration context, presenting verified clinical summaries directly to care team members.

24.17.10.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.17.10.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Resolve LHR Query within Collaboration (p. 765)	Established
From: associated feature	Service: Collaboration Clinical Summary Resolution (p. 766)	Established

24.17.10.10. Feature: Zero-PHI Collaboration Metadata Governance

Property	Value	Meaning
Reference Id	`FEAT-ISE-18`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-18`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration → BEC CT3: Clinical Context Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-ISE-18`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Clinical Collaboration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:645 (p. 1381)	Source standing does not approve the derived assertion.

24.17.10.10.1. Feature Overview

Metadata governance keeps patient names, identifiers and sensitive detail out of collaboration titles, topics and aliases. Opaque references preserve useful contextual association without placing PHI in broadly visible channel metadata or diagnostic surfaces.

24.17.10.10.2. Definition / governed semantic detail

Ensure collaboration channel titles, topics, and aliases use opaque identifiers free of patient names, MRNs, or sensitive details.

24.17.10.10.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.17.10.10.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Collaboration Metadata (p. 766)	Established
From: associated feature	Service: Collaboration Space Governance Service (p. 767)	Established

24.17.10.11. Feature: Collaboration Interaction Capture Qualification

Property	Value	Meaning
Reference Id	`HCM-FT-collaboration-interaction-capture-qualification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-STEP2-COLLABORATION-INTERACTION-CAPTURE-QUALIFICATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration → BEC CT3: Clinical Context Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-STEP2-COLLABORATION-INTERACTION-CAPTURE-QUALIFICATION`	Direct to this identity; never reusable.
Evidence	agora-omilia-collaboration.md:25; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.17.10.11.1. Feature Overview

A message may convey existing information or an explicit interaction may derive a new managed consequence. The distinction preserves collaboration-native records while routing governed consequences to their accountable responsibility.

24.17.10.11.2. Definition — Proposed

Qualify when a defined collaboration interaction creates a new governed information or activity consequence.

24.17.10.11.3. Boundary

Conversation presence, display or read receipt does not itself make a clinical decision, approval or final record.

24.17.10.11.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.17.10.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task agora-omilia-collaboration.md:25; information-responsibility.md:10

24.17.11. Function: Coordinate Care-Team Collaboration

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-care-team-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-COORDINATE-CARE-TEAM-COLLABORATION`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FN-COORDINATE-CARE-TEAM-COLLABORATION`	Direct to this identity; never reusable.
Evidence	ise:146 (p. 1469); metamodel:175 (p. 1390)	Source standing does not approve the derived assertion.

24.17.11.1. Function Overview

This Function maintains clinical collaboration spaces and participant context, enabling team discourse around care. The governed source also says it manages clinical handovers, which conflicts with the metamodel's explicit exclusion of that authority. The conflict is preserved for adjudication; this explanatory catalogue does not resolve it by quietly deleting either statement.

24.17.11.2. Definition / governed semantic detail

Manages care-team communication spaces, participant memberships, and clinical handovers.

24.17.11.3. Preserved v0.1 Review Matters

An existing description appears to conflict with a governing responsibility boundary; preserved for architectural review under AX-17 without reinterpretation.

24.17.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Care-Team Clinical Collaboration Coordination (p. 759)	Established

24.17.12. Service: Clinical Collaboration Space Service

Property	Value	Meaning
Reference Id	`HCM-SV-clinical-collaboration-space-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-CLINICAL-COLLABORATION-SPACE-SERVICE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.SV-CLINICAL-COLLABORATION-SPACE-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:147 (p. 1469)	Source standing does not approve the derived assertion.

24.17.12.1. Definition / governed semantic detail

Manages team collaboration spaces and discussions.

24.17.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Care-Team Clinical Collaboration Coordination (p. 759)	Established

24.17.13. Function: Resolve LHR Query within Collaboration

Property	Value	Meaning
Reference Id	`HCM-FN-resolve-lhr-query-within-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-RESOLVE-LHR-QUERY-WITHIN-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FN-RESOLVE-LHR-QUERY-WITHIN-COLLABORATION`	Direct to this identity; never reusable.
Evidence	ise:149 (p. 1469)	Source standing does not approve the derived assertion.

24.17.13.1. Definition / governed semantic detail

Contextually projects active patient summaries directly into collaborative clinical discussion channels.

24.17.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: In-Conversation LHR Query Resolution (p. 760)	Established

24.17.14. Service: Collaboration Clinical Summary Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-collaboration-clinical-summary-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-CLINICAL-SUMMARY-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-CLINICAL-SUMMARY-RESOLUTION`	Direct to this identity; never reusable.
Evidence	ise:150 (p. 1469)	Source standing does not approve the derived assertion.

24.17.14.1. Definition / governed semantic detail

Embeds clinical summaries in collaboration feeds.

24.17.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: In-Conversation LHR Query Resolution (p. 760)	Established

24.17.15. Function: Govern Collaboration Metadata

Property	Value	Meaning
Reference Id	`HCM-FN-govern-collaboration-metadata`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-GOVERN-COLLABORATION-METADATA`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FN-GOVERN-COLLABORATION-METADATA`	Direct to this identity; never reusable.
Evidence	ise:154 (p. 1469)	Source standing does not approve the derived assertion.

24.17.15.1. Definition / governed semantic detail

Enforces privacy policies, participant access gates, and zero-PHI space metadata rules.

24.17.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	Established

24.17.16. Service: Collaboration Space Governance Service

Property	Value	Meaning
Reference Id	`HCM-SV-collaboration-space-governance-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-SPACE-GOVERNANCE-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-SPACE-GOVERNANCE-SERVICE`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	ise:155 (p. 1469)	Source standing does not approve the derived assertion.

24.17.16.1. Definition / governed semantic detail

Governs space lifecycles and membership.

24.17.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	Established

24.18. BEC CT2: General Collaboration

Property	Value	Meaning
Reference Id	`HCM-BEC-general-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:572 (p. 1379); enabling:647 (p. 1381); ise:161 (p. 1469)	Source standing does not approve the derived assertion.

24.18.1. Capability Overview

General Collaboration provides secure operational messaging for departmental, administrative and facility-support participants. It gives non-clinical coordination an interaction context without assuming every discussion requires a patient-linked clinical workspace. Its role is communication enablement rather than ownership of the support activity being discussed.

24.18.2. Definition — Reviewed candidate

General Collaboration provides secure operational messaging for departmental, administrative and facility-support participants. It gives non-clinical coordination an interaction context without assuming every discussion requires a patient-linked clinical workspace. Its role is communication enablement rather than ownership of the support activity being discussed.

24.18.3. Boundary — retained v0.1 source explanation

Conversation does not assign operational authority, complete work or become an authoritative record merely through retention.

24.18.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Operational messaging materially enables coordination while the participants' business responsibilities remain external to the collaboration mechanism.

24.18.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.18.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.18.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.18.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Coordinate Operational Collaboration (p. 772)	function	Canonical / Established
Service: Operational Collaboration Service (p. 772)	service	Canonical / Established
BEC CT3: Operational Collaboration (p. 770)	capability CT3	Proposed / Proposed

24.18.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Digital Communication Channel (p. 138)	Proposed
To: enterprise enabled by	Enterprise CT2: Interaction & Experience (p. 862)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

24.18.10. BEC CT3: Operational Collaboration

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-operational-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.CT3-BEC-OPERATIONAL-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: General Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.18.10.1. Capability Overview

Enable operational team discussion and collaboration without promoting conversational content to clinical record authority. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.18.10.2. Definition — Reviewed candidate

Enable operational team discussion and collaboration without promoting conversational content to clinical record authority.

24.18.10.3. Intent

Enable operational team discussion and collaboration without promoting conversational content to clinical record authority.

24.18.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.18.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.18.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-2D46A392` (line 9322). Working locators are not identity aliases.

24.18.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Operational Team Chat Enablement (p. 771)	feature FT	Canonical / Proposed

24.18.10.8. Feature: Operational Team Chat Enablement

Property	Value	Meaning
Reference Id	`FEAT-ISE-19`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.CT3-BEC-OPERATIONAL-COLLABORATION.FEAT-ISE-19`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: General Collaboration → BEC CT3: Operational Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION.FEAT-ISE-19`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: General Collaboration	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:649 (p. 1381)	Source standing does not approve the derived assertion.

24.18.10.8.1. Feature Overview

Operational chat provides secure communication for administrative and support teams. It enables participants to discuss work while leaving assignment, completion and the authority of operational decisions with the capabilities responsible for that activity.

24.18.10.8.2. Definition / governed semantic detail

Provide secure instant messaging channels for departmental, administrative, and facility support staff.

24.18.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.18.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Coordinate Operational Collaboration (p. 772)	Established
From: associated feature	Service: Operational Collaboration Service (p. 772)	Established

24.18.11. Function: Coordinate Operational Collaboration

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-operational-collaboration`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.FN-COORDINATE-OPERATIONAL-COLLABORATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: General Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION.FN-COORDINATE-OPERATIONAL-COLLABORATION`	Direct to this identity; never reusable.
Evidence	ise:164 (p. 1469)	Source standing does not approve the derived assertion.

24.18.11.1. Definition / governed semantic detail

Provides communication spaces for facility logistics, disaster management teams, and administrative committees.

24.18.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Operational Team Chat Enablement (p. 771)	Established

24.18.12. Service: Operational Collaboration Service

Property	Value	Meaning
Reference Id	`HCM-SV-operational-collaboration-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.SV-OPERATIONAL-COLLABORATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: General Collaboration	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION.SV-OPERATIONAL-COLLABORATION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:165 (p. 1469)	Source standing does not approve the derived assertion.

24.18.12.1. Definition / governed semantic detail

Provides non-clinical operational messaging channels.

24.18.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Operational Team Chat Enablement (p. 771)	Established

24.19. BEC CT2: Workflow & Activity Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-workflow-activity-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-WORKFLOW-ACTIVITY-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.19.1. Capability Overview

Coordinate tasks entering or leaving the Harmonia execution boundary and work undertaken by other systems, with responsibility/destination routing, external status/traceability and governance of the completion bridge. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.19.2. Definition — Reviewed candidate

Coordinate tasks entering or leaving the Harmonia execution boundary and work undertaken by other systems, with responsibility/destination routing, external status/traceability and governance of the completion bridge.

24.19.3. Intent

Coordinate tasks entering or leaving the Harmonia execution boundary and work undertaken by other systems, with responsibility/destination routing, external status/traceability and governance of the completion bridge.

24.19.4. Boundary

External means outside Harmonia execution, potentially inside the same health organisation. Work coordination includes completion responsibilities and traceability; it is not generic interoperability or message routing.

24.19.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.19.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: External Task Coordination (p. 774)	capability CT3	Proposed / Proposed
BEC CT3: External Task Status & Traceability Management (p. 775)	capability CT3	Proposed / Proposed
BEC CT3: Activity Routing Management (p. 777)	capability CT3	Proposed / Proposed
BEC CT3: Work Boundary Governance (p. 778)	capability CT3	Proposed / Proposed

24.19.7. BEC CT3: External Task Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-external-task-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-WORKFLOW-ACTIVITY-COORDINATION.CT3-BEC-EXTERNAL-TASK-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.19.7.1. Capability Overview

Coordinate work entering or leaving the Harmonia execution boundary with the system or actor responsible for undertaking and completing that work. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.19.7.2. Definition — Reviewed candidate

Coordinate work entering or leaving the Harmonia execution boundary with the system or actor responsible for undertaking and completing that work.

24.19.7.3. Intent

Coordinate work entering or leaving the Harmonia execution boundary with the system or actor responsible for undertaking and completing that work.

24.19.7.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.19.7.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.19.8. BEC CT3: External Task Status & Traceability Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-external-task-status-traceability-management`	Stable concept identity.

Property	Value	Meaning
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-WORKFLOW-ACTIVITY-COORDINATION.CT3-BEC-EXTERNAL-TASK-STATUS-TRACEABILITY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.19.8.1. Capability Overview

Manage the observable status and attributable traceability of work as it progresses through systems outside the Harmonia execution boundary. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.19.8.2. Definition — Reviewed candidate

Manage the observable status and attributable traceability of work as it progresses through systems outside the Harmonia execution boundary.

24.19.8.3. Intent

Manage the observable status and attributable traceability of work as it progresses through systems outside the Harmonia execution boundary.

24.19.8.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.19.8.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.19.9. BEC CT3: Activity Routing Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-activity-routing-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-WORKFLOW-ACTIVITY-COORDINATION.CT3-BEC-ACTIVITY-ROUTING-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.19.9.1. Capability Overview

Manage responsibility and destination routing for activity crossing the Harmonia execution boundary, respecting the intended work and completion responsibility. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.19.9.2. Definition — Reviewed candidate

Manage responsibility and destination routing for activity crossing the Harmonia execution boundary, respecting the intended work and completion responsibility.

24.19.9.3. Intent

Manage responsibility and destination routing for activity crossing the Harmonia execution boundary, respecting the intended work and completion responsibility.

24.19.9.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.19.9.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.19.10. BEC CT3: Work Boundary Governance

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-work-boundary-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-WORKFLOW-ACTIVITY-COORDINATION.CT3-BEC-WORK-BOUNDARY-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.19.10.1. Capability Overview

Govern boundary conditions and completion responsibilities of the bridge between Harmonia and another system undertaking work. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.19.10.2. Definition — Reviewed candidate

Govern boundary conditions and completion responsibilities of the bridge between Harmonia and another system undertaking work.

24.19.10.3. Intent

Govern boundary conditions and completion responsibilities of the bridge between Harmonia and another system undertaking work.

24.19.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.19.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.20. BEC CT2: Calendar Management

Property	Value	Meaning
Reference Id	`HCM-BEC-calendar-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:574 (p. 1379); enabling:658 (p. 1381); ise:200 (p. 1470)	Source standing does not approve the derived assertion.

24.20.1. Capability Overview

Calendar Management projects schedules and correlates events with temporal commitments. It brings rosters, on-call windows, facility calendars and appointment milestones into operational views without replacing their authoritative sources. This distinguishes the temporal context used for coordination from the business decisions that establish the schedule.

24.20.2. Definition — Reviewed candidate

Calendar Management projects schedules and correlates events with temporal commitments. It brings rosters, on-call windows, facility calendars and appointment milestones into operational views without replacing their authoritative sources. This distinguishes the temporal context used for coordination from the business decisions that establish the schedule.

24.20.3. Boundary — retained v0.1 source explanation

Projection does not author the roster or confer qualification, privilege or clinical assignment authority.

24.20.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Consistent temporal projections and event correlation are direct platform coordination responsibilities.

24.20.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.20.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.20.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.20.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Project Operational Schedule (p. 783)	function	Canonical / Established
Service: Unified Calendar Projection Query (p. 784)	service	Canonical / Established
Function: Correlate Activity with Temporal Context (p. 785)	function	Canonical / Established
Service: Temporal Context Resolution (p. 785)	service	Canonical / Established
BEC CT3: Operational & Clinical Calendar Coordination (p. 780)	capability CT3	Proposed / Proposed

24.20.9. BEC CT3: Operational & Clinical Calendar Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-operational-clinical-calendar-coordination`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.20.9.1. Capability Overview

Coordinate projections of operational and clinical calendars and correlate temporal events without taking ownership of source bookings or rosters. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.20.9.2. Definition — Reviewed candidate

Coordinate projections of operational and clinical calendars and correlate temporal events without taking ownership of source bookings or rosters.

24.20.9.3. Intent

Coordinate projections of operational and clinical calendars and correlate temporal events without taking ownership of source bookings or rosters.

24.20.9.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.20.9.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.20.9.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-8CD7EA17` (line 9535). Working locators are not identity aliases.

24.20.9.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Schedule Projection Integration (p. 781)	feature FT	Canonical / Proposed
Feature: Temporal Event Correlation (p. 782)	feature FT	Canonical / Proposed

24.20.9.8. Feature: Schedule Projection Integration

Property	Value	Meaning
Reference Id	`FEAT-ISE-24`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION.FEAT-ISE-24`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management → BEC CT3: Operational & Clinical Calendar Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FEAT-ISE-24`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Calendar Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:660 (p. 1382)	Source standing does not approve the derived assertion.

24.20.9.8.1. Feature Overview

Schedule projection makes external roster, on-call and facility calendars available in operational views. It preserves their temporal usefulness without making the projection the authoritative schedule or granting authority to alter staffing and coverage decisions.

24.20.9.8.2. Definition / governed semantic detail

Project clinician rosters, on-call schedules, and facility operating calendars into operational views.

24.20.9.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.20.9.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Project Operational Schedule (p. 783)	Established
From: associated feature	Service: Unified Calendar Projection Query (p. 784)	Established

24.20.9.9. Feature: Temporal Event Correlation

Property	Value	Meaning
Reference Id	`FEAT-ISE-25`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION.FEAT-ISE-25`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management → BEC CT3: Operational & Clinical Calendar Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FEAT-ISE-25`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Calendar Management	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:661 (p. 1382)	Source standing does not approve the derived assertion.

24.20.9.9.1. Feature Overview

Temporal correlation relates incoming events to the relevant appointment or calendar milestone. Association provides context for interpretation and coordination without assuming that temporal proximity proves attendance, fulfilment or the clinical outcome expected at that time.

24.20.9.9.2. Definition / governed semantic detail

Correlate incoming clinical events with scheduled appointments and operational calendar milestones.

24.20.9.9.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.20.9.9.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Correlate Activity with Temporal Context (p. 785)	Established
From: associated feature	Service: Temporal Context Resolution (p. 785)	Established

24.20.10. Function: Project Operational Schedule

Property	Value	Meaning
Reference Id	`HCM-FN-project-operational-schedule`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.FN-PROJECT-OPERATIONAL-SCHEDULE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FN-PROJECT-OPERATIONAL-SCHEDULE`	Direct to this identity; never reusable.
Evidence	ise:205 (p. 1470)	Source standing does not approve the derived assertion.

24.20.10.1. Definition / governed semantic detail

Aggregates and overlays multidisciplinary schedules onto a unified temporal view.

24.20.10.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Schedule Projection Integration (p. 781)	Established

24.20.11. Service: Unified Calendar Projection Query

Property	Value	Meaning
Reference Id	`HCM-SV-unified-calendar-projection-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.SV-UNIFIED-CALENDAR-PROJECTION-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.SV-UNIFIED-CALENDAR-PROJECTION-QUERY`	Direct to this identity; never reusable.
Evidence	ise:206 (p. 1470)	Source standing does not approve the derived assertion.

24.20.11.1. Definition / governed semantic detail

Discloses consolidated temporal schedules.

24.20.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Schedule Projection Integration (p. 781)	Established

24.20.12. Function: Correlate Activity with Temporal Context

Property	Value	Meaning
Reference Id	`HCM-FN-correlate-activity-with-temporal-context`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.FN-CORRELATE-ACTIVITY-WITH-TEMPORAL-CONTEXT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FN-CORRELATE-ACTIVITY-WITH-TEMPORAL-CONTEXT`	Direct to this identity; never reusable.
Evidence	ise:208 (p. 1470)	Source standing does not approve the derived assertion.

24.20.12.1. Definition / governed semantic detail

Associates clinical events with shift boundaries and appointment slots.

24.20.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Temporal Event Correlation (p. 782)	Established

24.20.13. Service: Temporal Context Resolution

Property	Value	Meaning
Reference Id	`HCM-SV-temporal-context-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.SV-TEMPORAL-CONTEXT-RESOLUTION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Calendar Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.SV-TEMPORAL-CONTEXT-RESOLUTION`	Direct to this identity; never reusable.
Evidence	ise:209 (p. 1470)	Source standing does not approve the derived assertion.

24.20.13.1. Definition / governed semantic detail

Resolves operational shifts.

24.20.13.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Temporal Event Correlation (p. 782)	Established

24.21. BEC CT2: Presentation Services

Property	Value	Meaning
Reference Id	`HCM-BEC-presentation-services`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.

Property	Value	Meaning
Evidence	enabling:575 (p. 1379); enabling:663 (p. 1382); ise:215 (p. 1471)	Source standing does not approve the derived assertion.

24.21.1. Capability Overview

Presentation Services supplies contextual human interaction through clinical timelines, directory views and operational dashboards. It makes governed information usable by people while consuming the identity, security and clinical context established elsewhere. The responsibility is the interaction and presentation experience rather than a new authority for the information shown.

24.21.2. Definition — Reviewed candidate

Presentation Services supplies contextual human interaction through clinical timelines, directory views and operational dashboards. It makes governed information usable by people while consuming the identity, security and clinical context established elsewhere. The responsibility is the interaction and presentation experience rather than a new authority for the information shown.

24.21.3. Boundary — retained v0.1 source explanation

Presentation does not own authentication, credential storage, consent decisions, EMPI reconciliation or authoritative clinical state. A displayed value is not made authoritative by its appearance in a dashboard.

24.21.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Decoupled contextual interaction is a substantive Core responsibility that preserves the established information-access and control boundaries.

24.21.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.21.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.21.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.21.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Present Contextual Health and Operational Information (p. 790)	function	Canonical / Established
Service: Contextual Presentation Service (p. 791)	service	Canonical / Established
BEC CT3: Contextual Information Presentation (p. 788)	capability CT3	Proposed / Proposed

24.21.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: enterprise enabled by	Enterprise CT2: Interaction & Experience (p. 862)	Proposed
To: enterprise enabled by	Enterprise CT2: Policy & Control (p. 865)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed

24.21.10. BEC CT3: Contextual Information Presentation

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-contextual-information-presentation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.CT3-BEC-CONTEXTUAL-INFORMATION-PRESENTATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Presentation Services	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.21.10.1. Capability Overview

Present governed longitudinal clinical and operational information in its applicable human interaction context. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.21.10.2. Definition — Reviewed candidate

Present governed longitudinal clinical and operational information in its applicable human interaction context.

24.21.10.3. Intent

Present governed longitudinal clinical and operational information in its applicable human interaction context.

24.21.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.21.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.21.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-2DCF9553` (line 9623). Working locators are not identity aliases.

24.21.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Longitudinal Clinical & Operational Presentation (p. 789)	feature FT	Canonical / Proposed

24.21.10.8. Feature: Longitudinal Clinical & Operational Presentation

Property	Value	Meaning
Reference Id	`FEAT-ISE-26`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.CT3-BEC-CONTEXTUAL-INFORMATION-PRESENTATION.FEAT-ISE-26`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Presentation Services → BEC CT3: Contextual Information Presentation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES.FEAT-ISE-26`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Presentation Services	Preserved accountability; narrower CT3 frame Proposed.

Property	Value	Meaning
Evidence	enabling:665 (p. 1382)	Source standing does not approve the derived assertion.

24.21.10.8.1. Feature Overview

Contextual presentation makes clinical timelines, directories and operational information usable for people. It consumes established identity and security context while preserving the originating authority of what is shown; a user interface does not become an alternative persistence or policy boundary.

24.21.10.8.2. Definition / governed semantic detail

Deliver contextual clinical timelines, directory administration views, and operational management dashboards.

24.21.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.21.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Present Contextual Health and Operational Information (p. 790)	Established
From: associated feature	Service: Contextual Presentation Service (p. 791)	Established

24.21.11. Function: Present Contextual Health and Operational Information

Property	Value	Meaning
Reference Id	`HCM-FN-present-contextual-health-and-operational-information`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.FN-PRESENT-CONTEXTUAL-HEALTH-AND-OPERATIONAL-INFORMATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Presentation Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES.FN-PRESENT-CONTEXTUAL-HEALTH-AND-OPERATIONAL-INFORMATION`	Direct to this identity; never reusable.
Evidence	ise:220 (p. 1471)	Source standing does not approve the derived assertion.

24.21.11.1. Definition / governed semantic detail

Composes clinical summaries, work queues, and patient records into responsive role-based user interfaces.

24.21.11.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Longitudinal Clinical & Operational Presentation (p. 789)	Established

24.21.12. Service: Contextual Presentation Service

Property	Value	Meaning
Reference Id	`HCM-SV-contextual-presentation-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.SV-CONTEXTUAL-PRESENTATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Presentation Services	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES.SV-CONTEXTUAL-PRESENTATION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:221 (p. 1471)	Source standing does not approve the derived assertion.

24.21.12.1. Definition / governed semantic detail

Delivers contextual health and operational information for presentation.

24.21.12.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Longitudinal Clinical & Operational Presentation (p. 789)	Established

24.22. BEC CT2: Information Design Governance

Property	Value	Meaning
Reference Id	`HCM-BEC-information-design-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE`	Direct to this identity; never reusable.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:576 (p. 1379); enabling:667 (p. 1382); ise:227 (p. 1471)	Source standing does not approve the derived assertion.

24.22.1. Capability Overview

Information Design Governance maintains canonical definitions, semantic bindings, exchange profiles and the conformance rules applied to representations. It makes meaning and constraints available across regional information flows so that an accepted technical format is not mistaken for semantic correctness. Governance and verification remain distinguishable from the clinical facts represented.

24.22.2. Definition — Reviewed candidate

Information Design Governance maintains canonical definitions, semantic bindings, exchange profiles and the conformance rules applied to representations. It makes meaning and constraints available across regional information flows so that an accepted technical format is not mistaken for semantic correctness. Governance and verification remain distinguishable from the clinical facts represented.

24.22.3. Boundary — retained v0.1 source explanation

Consumers of definitions do not acquire their semantic authority, and conformance checking does not establish clinical truth or permission to disclose.

24.22.4. Harmonia Architectural Relevance — retained v0.1 source explanation

Governed semantics and representation conformance are direct platform responsibilities that constrain both internal use and boundary exchange.

24.22.5. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

24.22.6. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

24.22.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.22.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Govern Information Semantics (p. 803)	function	Canonical / Established
Service: Semantic Model Definition Service (p. 804)	service	Canonical / Established
Function: Govern Information Definition (p. 804)	function	Canonical / Established
Service: Information Dictionary Query (p. 805)	service	Canonical / Established
Function: Govern Terminology Binding (p. 806)	function	Canonical / Established
Service: Terminology Binding Specification Lookup (p. 806)	service	Canonical / Established
Function: Govern Exchange Profile (p. 807)	function	Canonical / Established
Service: Exchange Profile Specification Query (p. 808)	service	Canonical / Established
Function: Verify Semantic Conformance (p. 808)	function	Canonical / Established
Service: Semantic Conformance Verification Service (p. 809)	service	Canonical / Established
BEC CT3: Enterprise Canonical/Reusable Model Design (p. 794)	capability CT3	Proposed / Proposed
BEC CT3: Enterprise Canonical/Reusable Model Library Management (p. 799)	capability CT3	Proposed / Proposed
BEC CT3: Enterprise Canonical/Reusable Model Utilisation Support (p. 800)	capability CT3	Proposed / Proposed
BEC CT3: Enterprise Reference Data Stewardship (p. 802)	capability CT3	Proposed / Proposed

24.22.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health Information Standards Governance (p. 121)	Proposed
From: enabled by	BC CT2: Clinical Semantics Governance (p. 128)	Proposed
To: enterprise enabled by	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed
To: enterprise enabled by	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed
To: enterprise enabled by	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed

24.22.10. BEC CT3: Enterprise Canonical/Reusable Model Design

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-enterprise-canonical-reusable-model-design`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.22.10.1. Capability Overview

Design canonical and reusable enterprise information models and semantic bindings under applicable information-design governance. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.22.10.2. Definition — Reviewed candidate

Design canonical and reusable enterprise information models and semantic bindings under applicable information-design governance.

24.22.10.3. Intent

Design canonical and reusable enterprise information models and semantic bindings under applicable information-design governance.

24.22.10.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.22.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.22.10.6. Prior Candidate Derivation

v0.3 reconciliation: `BEC-453A5B71` (line 9699). Working locators are not identity aliases.

24.22.10.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Information Standards & Semantic Governance (p. 795)	feature FT	Canonical / Proposed
Feature: Information Definition & Schema Governance (p. 796)	feature FT	Proposed / Proposed
Feature: Terminology Binding Governance (p. 797)	feature FT	Proposed / Proposed
Feature: Exchange Profile Governance (p. 798)	feature FT	Proposed / Proposed

24.22.10.8. Feature: Information Standards & Semantic Governance

Property	Value	Meaning
Reference Id	`FEAT-ISE-27`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-ISE-27`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance → BEC CT3: Enterprise Canonical/Reusable Model Design	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-ISE-27`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Information Design Governance	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:669 (p. 1382)	Source standing does not approve the derived assertion.

24.22.10.8.1. Feature Overview

Semantic governance provides canonical definitions, bindings and profiles through which regional representations can be understood consistently. Its purpose concerns meaning and constraints rather than a particular schema technology, and consuming a definition does not transfer its governing authority.

24.22.10.8.2. Definition / governed semantic detail

Govern canonical clinical data definitions, terminology bindings, and exchange profiles across regional systems.

24.22.10.8.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.22.10.8.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Information Semantics (p. 803)	Established
From: associated feature	Service: Semantic Model Definition Service (p. 804)	Established

24.22.10.9. Feature: Information Definition & Schema Governance

Property	Value	Meaning
Reference Id	`HCM-FT-information-definition-schema-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-INFORMATION-DEFINITION-SCHEMA-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance → BEC CT3: Enterprise Canonical/Reusable Model Design	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-STEP2-INFORMATION-DEFINITION-SCHEMA-GOVERNANCE`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10; 05-intrinsic-enablement.md:234	Source standing does not approve the derived assertion.

24.22.10.9.1. Feature Overview

Existing definition-governance behaviour needs a distinct Feature frame from general semantic-model authorship and runtime validation.

24.22.10.9.2. Definition — Proposed

Govern structural definitions, data dictionaries and constraint meaning.

24.22.10.9.3. Boundary

Governed definitions do not own runtime instances or their clinical facts.

24.22.10.9.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.22.10.9.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10; 05-intrinsic-enablement.md:234

24.22.10.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Information Definition (p. 804)	Proposed

24.22.10.10. Feature: Terminology Binding Governance

Property	Value	Meaning
Reference Id	`HCM-FT-terminology-binding-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-TERMINOLOGY-BINDING-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance → BEC CT3: Enterprise Canonical/Reusable Model Design	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-STEP2-TERMINOLOGY-BINDING-GOVERNANCE`	Direct to this identity; never reusable.
Evidence	calliope-models-and-datasets.md:120; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.22.10.10.1. Feature Overview

Binding a data element to a vocabulary is distinct from resolving a patient code or changing the runtime membership of a dataset.

24.22.10.10.2. Definition — Proposed

Govern how information definitions are bound to applicable terminology and value sets.

24.22.10.10.3. Boundary

Definition of a binding does not take external ontology authority or execute policy.

24.22.10.10.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.22.10.10.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task calliope-models-and-datasets.md:120; information-responsibility.md:10

24.22.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Terminology Binding (p. 806)	Proposed

24.22.10.11. Feature: Exchange Profile Governance

Property	Value	Meaning
Reference Id	`HCM-FT-exchange-profile-governance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-EXCHANGE-PROFILE-GOVERNANCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance → BEC CT3: Enterprise Canonical/Reusable Model Design	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-STEP2-EXCHANGE-PROFILE-GOVERNANCE`	Direct to this identity; never reusable.
Evidence	architectural-axioms.md:613; information-responsibility.md:10	Source standing does not approve the derived assertion.

24.22.10.11.1. Feature Overview

An external contract must state which representations and extensions are permitted; validation applies that contract but does not define it.

24.22.10.11.2. Definition — Proposed

Govern the semantic and conformance constraints of an interoperability profile.

24.22.10.11.3. Boundary

Profile governance does not transmit messages or publish internal operational metadata.

24.22.10.11.4. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.22.10.11.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:613; information-responsibility.md:10

24.22.10.11.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Govern Exchange Profile (p. 807)	Proposed

24.22.11. BEC CT3: Enterprise Canonical/Reusable Model Library Management

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-enterprise-canonical-reusable-model-library-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-LIBRARY-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.22.11.1. Capability Overview

Manage the identifiable library of canonical and reusable enterprise model definitions and their governed revisions. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.22.11.2. Definition — Reviewed candidate

Manage the identifiable library of canonical and reusable enterprise model definitions and their governed revisions.

24.22.11.3. Intent

Manage the identifiable library of canonical and reusable enterprise model definitions and their governed revisions.

24.22.11.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.22.11.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.22.12. BEC CT3: Enterprise Canonical/Reusable Model Utilisation Support

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-enterprise-canonical-reusable-model-utilisation-support`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-UTILISATION-SUPPORT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.22.12.1. Capability Overview

Support correct utilisation and conformance of canonical and reusable models across their intended consumers. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.22.12.2. Definition — Reviewed candidate

Support correct utilisation and conformance of canonical and reusable models across their intended consumers.

24.22.12.3. Intent

Support correct utilisation and conformance of canonical and reusable models across their intended consumers.

24.22.12.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.22.12.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.22.12.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Feature: Semantic Conformance Verification (p. 801)	feature FT	Canonical / Proposed

24.22.12.7. Feature: Semantic Conformance Verification

Property	Value	Meaning
Reference Id	`FEAT-ISE-28`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-UTILISATION-SUPPORT.FEAT-ISE-28`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance → BEC CT3: Enterprise Canonical/Reusable Model Utilisation Support	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-ISE-28`	Direct to this identity; never reusable.
Established broader owner	BEC CT2: Information Design Governance	Preserved accountability; narrower CT3 frame Proposed.
Evidence	enabling:670 (p. 1382)	Source standing does not approve the derived assertion.

24.22.12.7.1. Feature Overview

Conformance verification evaluates a representation against published semantic rules and constraints. It distinguishes meaningful conformity from simple parseability while retaining that a conformant message is not necessarily clinically true, authoritative or permitted for disclosure.

24.22.12.7.2. Definition / governed semantic detail

Verify inbound and outbound representations against published semantic rules and conformance constraints.

24.22.12.7.3. Proposed Placement Rationale

Meaningful reviewed CT3 framing selected for the existing Feature semantics; established responsibility provenance is preserved.

24.22.12.7.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Verify Semantic Conformance (p. 808)	Established
From: associated feature	Service: Semantic Conformance Verification Service (p. 809)	Established

24.22.13. BEC CT3: Enterprise Reference Data Stewardship

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-enterprise-reference-data-stewardship`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-REFERENCE-DATA-STEWARDSHIP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

24.22.13.1. Capability Overview

Steward enterprise reference-data definitions, bindings and applicable governance independently of mutable runtime dataset administration. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

24.22.13.2. Definition — Reviewed candidate

Steward enterprise reference-data definitions, bindings and applicable governance independently of mutable runtime dataset administration.

24.22.13.3. Intent

Steward enterprise reference-data definitions, bindings and applicable governance independently of mutable runtime dataset administration.

24.22.13.4. Boundary

Information administration and coordination preserve the authority of the responsible source, clinical actor and governed domain; they confer no additional real-world management authority.

24.22.13.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

24.22.14. Function: Govern Information Semantics

Property	Value	Meaning
Reference Id	`HCM-FN-govern-information-semantics`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-SEMANTICS`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-SEMANTICS`	Direct to this identity; never reusable.
Evidence	ise:231 (p. 1471)	Source standing does not approve the derived assertion.

24.22.14.1. Definition / governed semantic detail

Authors and validates canonical data concepts and entity relationships.

24.22.14.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Information Standards & Semantic Governance (p. 795)	Established

24.22.15. Service: Semantic Model Definition Service

Property	Value	Meaning
Reference Id	`HCM-SV-semantic-model-definition-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-MODEL-DEFINITION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-MODEL-DEFINITION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:232 (p. 1471)	Source standing does not approve the derived assertion.

24.22.15.1. Definition / governed semantic detail

Discloses canonical model definitions.

24.22.15.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Information Standards & Semantic Governance (p. 795)	Established

24.22.16. Function: Govern Information Definition

Property	Value	Meaning
Reference Id	`HCM-FN-govern-information-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-DEFINITION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-DEFINITION`	Direct to this identity; never reusable.
Evidence	ise:234 (p. 1471); ise:233 (p. 1471)	Source standing does not approve the derived assertion.

24.22.16.1. Definition / governed semantic detail

Maintains governed data element dictionaries, data types, and structural rules.

24.22.16.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Information Definition & Schema Governance (p. 796)	Proposed

24.22.17. Service: Information Dictionary Query

Property	Value	Meaning
Reference Id	`HCM-SV-information-dictionary-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-INFORMATION-DICTIONARY-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-INFORMATION-DICTIONARY-QUERY`	Direct to this identity; never reusable.
Evidence	ise:235 (p. 1471); ise:233 (p. 1471)	Source standing does not approve the derived assertion.

24.22.17.1. Definition / governed semantic detail

Provides data dictionary specifications.

24.22.18. Function: Govern Terminology Binding

Property	Value	Meaning
Reference Id	`HCM-FN-govern-terminology-binding`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-TERMINOLOGY-BINDING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-TERMINOLOGY-BINDING`	Direct to this identity; never reusable.
Evidence	ise:237 (p. 1471); ise:236 (p. 1471)	Source standing does not approve the derived assertion.

24.22.18.1. Definition / governed semantic detail

Binds canonical data elements to authoritative terminology value sets.

24.22.18.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Terminology Binding Governance (p. 797)	Proposed

24.22.19. Service: Terminology Binding Specification Lookup

Property	Value	Meaning
Reference Id	`HCM-SV-terminology-binding-specification-lookup`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-TERMINOLOGY-BINDING-SPECIFICATION-LOOKUP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-TERMINOLOGY-BINDING-SPECIFICATION-LOOKUP`	Direct to this identity; never reusable.
Evidence	ise:238 (p. 1471); ise:236 (p. 1471)	Source standing does not approve the derived assertion.

24.22.19.1. Definition / governed semantic detail

Discloses value set bindings.

24.22.20. Function: Govern Exchange Profile

Property	Value	Meaning
Reference Id	`HCM-FN-govern-exchange-profile`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-EXCHANGE-PROFILE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-EXCHANGE-PROFILE`	Direct to this identity; never reusable.
Evidence	ise:240 (p. 1471); ise:239 (p. 1471)	Source standing does not approve the derived assertion.

24.22.20.1. Definition / governed semantic detail

Defines standards-based interchange profiles and message constraints.

24.22.20.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Exchange Profile Governance (p. 798)	Proposed

24.22.21. Service: Exchange Profile Specification Query

Property	Value	Meaning
Reference Id	`HCM-SV-exchange-profile-specification-query`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-EXCHANGE-PROFILE-SPECIFICATION-QUERY`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-EXCHANGE-PROFILE-SPECIFICATION-QUERY`	Direct to this identity; never reusable.
Evidence	ise:241 (p. 1471); ise:239 (p. 1471)	Source standing does not approve the derived assertion.

24.22.21.1. Definition / governed semantic detail

Discloses interoperability schemas.

24.22.22. Function: Verify Semantic Conformance

Property	Value	Meaning
Reference Id	`HCM-FN-verify-semantic-conformance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-VERIFY-SEMANTIC-CONFORMANCE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-VERIFY-SEMANTIC-CONFORMANCE`	Direct to this identity; never reusable.
Evidence	ise:243 (p. 1471)	Source standing does not approve the derived assertion.

24.22.22.1. Definition / governed semantic detail

Validates candidate messages against canonical governance schemas.

24.22.22.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Semantic Conformance Verification (p. 801)	Established

24.22.23. Service: Semantic Conformance Verification Service

Property	Value	Meaning
Reference Id	`HCM-SV-semantic-conformance-verification-service`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-CONFORMANCE-VERIFICATION-SERVICE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Design Governance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-CONFORMANCE-VERIFICATION-SERVICE`	Direct to this identity; never reusable.
Evidence	ise:244 (p. 1471)	Source standing does not approve the derived assertion.

24.22.23.1. Definition / governed semantic detail

Validates representations for compliance.

24.22.23.2. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: associated feature	Feature: Semantic Conformance Verification (p. 801)	Established

25. BEC CT1: Health Informatics Management

Property	Value	Meaning
Reference Id	`HCM-BEC-secondary-health-information-use-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT`	Current proposed context where marked Proposed.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT`	Direct to this identity; never reusable.
Name history	Secondary Health Information Use Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

25.1. Capability Overview

Establish and govern semantically meaningful health-information assets suitable for downstream research, population-health reporting and service planning in an adjacent analytical environment.

25.2. Definition — Reviewed candidate

Establish and govern semantically meaningful health-information assets suitable for downstream research, population-health reporting and service planning in an adjacent analytical environment.

25.3. Boundary

Harmonia is adjacent to a Big Data/analytical environment. It establishes governed assets and reporting inputs; it does not become that analytical platform.

25.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

25.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

25.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

25.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

25.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Provide existing health information for authorised research, required population notification and service planning purposes.

Previous overview: Provide existing health information for authorised research, required population notification and service planning purposes. Its constituent abilities distinguish .

Previous intent: Preserve the particular authorised purpose and obligation when clinical information is used beyond individual care delivery.

Previous boundary: Each purpose has different release criteria. This grouping does not originate research approval, public-health decisions or planning authority.

25.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Research Information Curation (p. 812)	capability CT2	Proposed / Proposed
BEC CT2: Population Health Information Reporting (p. 817)	capability CT2	Proposed / Proposed
BEC CT2: Service Planning Data Management (p. 818)	capability CT2	Proposed / Proposed

25.10. BEC CT2: Research Information Curation

Property	Value	Meaning
Reference Id	`HCM-BEC-authorised-research-information-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Informatics Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION`	Direct to this identity; never reusable.
Name history	Authorised Research Information Provision	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:842; information-responsibility.md:10	Source standing does not approve the derived assertion.

25.10.1. Capability Overview

Establish and curate governed data assets suitable for explicitly authorised research use, retaining source attribution and applicable release constraints.

25.10.2. Definition — Reviewed candidate

Establish and curate governed data assets suitable for explicitly authorised research use, retaining source attribution and applicable release constraints.

25.10.3. Boundary

Does not grant ethics approval, recruit trial participants, manage trials or become the authority for derived research conclusions.

25.10.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

25.10.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

25.10.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:842; information-responsibility.md:10

25.10.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

25.10.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Assemble and release existing health information for an explicitly authorised research purpose under applicable use constraints.

Previous overview: Research support must retain the purpose, applicable approvals and release qualification of supplied information. It reuses governed retrieval and privacy controls rather than claiming a clinical-trial or research-governance platform.

Previous intent: Supply the permitted existing information for a particular authorised research use.

25.10.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Assemble Purpose-Qualified Research Release (p. 814)	function	Proposed / Proposed
Service: Authorised Research Information Export (p. 815)	service	Proposed / Proposed

25.10.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Research Information Support (p. 160)	Proposed
From: purpose context	Feature: Authorised Research Release Assembly (p. 739)	Proposed

25.10.11. Function: Assemble Purpose-Qualified Research Release

Property	Value	Meaning
Reference Id	`HCM-FN-assemble-purpose-qualified-research-release`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FN-ASSEMBLE-PURPOSE-QUALIFIED-RESEARCH-RELEASE`	Current proposed context where marked Proposed.

Property	Value	Meaning
Current context	BEC CT1: Health Informatics Management → BEC CT2: Research Information Curation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FN-ASSEMBLE-PURPOSE-QUALIFIED-RESEARCH-RELEASE`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10; business-capabilities.md:842	Source standing does not approve the derived assertion.

25.10.11.1. Function Overview

Moves a qualified request to a bounded release with source attribution and applicable use constraints.

25.10.11.2. Definition — Proposed

Construct a release containing only existing information permitted for the explicitly authorised research purpose.

25.10.11.3. Boundary

Does not author ethics approvals, promise de-identification or acquire ownership of research conclusions.

25.10.11.4. Proposed Placement Rationale

Behaviour remains within Authorised Research Information Provision responsibility.

25.10.11.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10; business-capabilities.md:842

25.10.11.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: realises feature	Feature: Authorised Research Release Assembly (p. 739)	Proposed
From: uses function	Service: Authorised Research Information Export (p. 815)	Proposed

25.10.12. Service: Authorised Research Information Export

Property	Value	Meaning
Reference Id	`HCM-SV-authorised-research-information-export`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Proposed	Step 2 architectural proposal.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.SV-AUTHORISED-RESEARCH-INFORMATION-EXPORT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Informatics Management → BEC CT2: Research Information Curation	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.SV-AUTHORISED-RESEARCH-INFORMATION-EXPORT`	Direct to this identity; never reusable.
Evidence	information-responsibility.md:10	Source standing does not approve the derived assertion.

25.10.12.1. Service Overview

The service exposes the permitted release and its applicable use basis rather than unrestricted clinical repository access.

25.10.12.2. Definition — Proposed

Provide an approved research consumer with purpose-qualified existing health information under its release contract.

25.10.12.3. Boundary

No new research-governance mandate or universal release format is allocated.

25.10.12.4. Proposed Placement Rationale

Behaviour remains within Authorised Research Information Provision responsibility.

25.10.12.5. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10

25.10.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: uses function	Function: Assemble Purpose-Qualified Research Release (p. 814)	Proposed

25.11. BEC CT2: Population Health Information Reporting

Property	Value	Meaning
Reference Id	`HCM-BEC-population-health-notification-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Informatics Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT`	Direct to this identity; never reusable.
Name history	Population Health Notification Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:611; information-responsibility.md:10	Source standing does not approve the derived assertion.

25.11.1. Capability Overview

Establish and provide governed information assets and reporting inputs for population-health purposes, including applicable notification obligations.

25.11.2. Definition — Reviewed candidate

Establish and provide governed information assets and reporting inputs for population-health purposes, including applicable notification obligations.

25.11.3. Boundary

Does not determine population-health findings, create statutory obligations or equate transport delivery with discharge of notification duty.

25.11.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

25.11.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

25.11.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:611; information-responsibility.md:10

25.11.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

25.11.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Prepare, convey and track required population-health notifications from applicable governed source information.

Previous overview: A reportable event and the obligation to notify are different from sending a message. This capability preserves the established obligation and observes the notification outcome under its contract without deciding public-health response.

Previous intent: Keep the applicable notification duty and the observed communication disposition traceable to each other.

25.11.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Population Health Surveillance (p. 94)	Proposed
To: enterprise enabled by	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed
To: enterprise enabled by	Enterprise CT2: Event & Subscription (p. 856)	Proposed
To: enterprise enabled by	Enterprise CT2: Operational Assurance (p. 873)	Proposed
From: purpose context	Feature: Population Notification Obligation Tracking (p. 740)	Proposed

25.12. BEC CT2: Service Planning Data Management

Property	Value	Meaning
Reference Id	`HCM-BEC-service-planning-information-provision`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-SERVICE-PLANNING-INFORMATION-PROVISION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Health Informatics Management	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-SERVICE-PLANNING-INFORMATION-PROVISION`	Direct to this identity; never reusable.
Name history	Service Planning Information Provision	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:626; information-responsibility.md:34	Source standing does not approve the derived assertion.

25.12.1. Capability Overview

Establish and maintain governed information assets suitable for health-service planning, modelling and analysis.

25.12.2. Definition — Reviewed candidate

Establish and maintain governed information assets suitable for health-service planning, modelling and analysis.

25.12.3. Boundary

Observed utilisation, operational capacity and planned capacity are distinct; information provision does not commission services or allocate resources.

25.12.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

25.12.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

25.12.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:626; information-responsibility.md:34

25.12.7. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

25.12.8. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Supply qualified demand, utilisation and capacity evidence to responsible health-service planning activities.

Previous overview: Planners need comparable time- and service-qualified observations from multiple operating settings. This ability prepares those projections while leaving the interpretation of required future capacity and distribution with the planning authority.

Previous intent: Give planners qualified observed evidence without predetermining the planning judgement.

25.12.9. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT2: Health-System Planning (p. 99)	Proposed
From: purpose context	Feature: Demand & Capacity Planning Projection (p. 741)	Proposed

26. BEC CT1: Independent Assurance Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-independent-service-assurance-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT`	Current proposed context where marked Proposed.
Name history	Independent Service Assurance Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; health-service-assurance.md:9	Source standing does not approve the derived assertion.

26.1. Capability Overview

Enable assurance specification and independent review of the assurance service while preserving the distinct Canonical responsibility for assurance against a governed subject.

26.2. Definition — Reviewed candidate

Enable assurance specification and independent review of the assurance service while preserving the distinct Canonical responsibility for assurance against a governed subject.

26.3. Boundary

Assurance cannot manage, remediate or approve its subject merely because it observes it. Clinical adequacy, financial governance and source-information authority remain outside this mandate.

26.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

26.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; health-service-assurance.md:9

26.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

26.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous definition: Specify how governed service behaviour is assured and independently establish what applicable evidence demonstrates.

Previous overview: The frame separates design and criteria stewardship from the evaluation activity that produces findings. It preserves the established three assurance capabilities and five Functions without turning observation or operational health into an assurance conclusion.

Previous intent: Establish justified confidence in governed service behaviour without giving the subject control of its own assurance conclusion.

26.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT2: Assurance Specification (p. 822)	capability CT2	Proposed / Proposed
BEC CT2: Assurance Service Review (p. 832)	capability CT2	Proposed / Proposed

26.9. BEC CT2: Assurance Specification

Property	Value	Meaning
Reference Id	`HCM-BEC-assurance-specification-enablement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Name history	Assurance Specification Enablement	Names do not establish another identity.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

26.9.1. Capability Overview

Model how satisfaction will be assured and maintain applicable, temporally qualified criteria.

26.9.2. Definition — Reviewed candidate

Model how satisfaction will be assured and maintain applicable, temporally qualified criteria.

26.9.3. Boundary

Modelling and criteria stewardship do not approve governing requirements or perform independent conclusion-making.

26.9.4. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

26.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

26.9.6. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

26.9.7. Pre-review semantics — historical

Exact prior assertions are retained as reconciliation evidence, rather than descriptions of the current reviewed capability.

Previous overview: Model how satisfaction will be assured and maintain applicable, temporally qualified criteria. Its constituent abilities distinguish Assurance Design; Assurance Criteria Management.

Previous intent: Make the intended basis of assurance explicit before evidence is assessed.

26.9.8. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Assurance Design (p. 824)	capability CT3	Canonical / Proposed
BEC CT3: Assurance Criteria Administration (p. 828)	capability CT3	Canonical / Proposed

26.9.9. BEC CT3: Assurance Design

Property	Value	Meaning
Reference Id	`HCM-BEC-assurance-design`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:680 (p. 1382)	Source standing does not approve the derived assertion.

26.9.9.1. Capability Overview

Model how satisfaction of a governed subject or requirement will be assured, preserving applicable assurance dispositions and boundaries.

26.9.9.2. Preserved Canonical semantic evidence — source context

The capability to determine and define how satisfaction of a governed requirement, constraint or expected behaviour will be assured, including its assurance disposition, applicable criteria, required evidence and evaluation expectations.

26.9.9.3. Definition — Reviewed candidate

Model how satisfaction of a governed subject or requirement will be assured, preserving applicable assurance dispositions and boundaries.

26.9.9.4. Boundary — retained v0.1 source explanation

Designing assurance does not confer approval authority, define the underlying obligation or allocate a runtime component. No Feature decomposition or CT ancestry is established.

26.9.9.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Formal classification remains Unresolved. Its approved support to Health Service Assurance establishes purpose and Enterprise Capability contributions without adjudicating this separate relevance dimension.

26.9.9.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

26.9.9.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

26.9.9.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

26.9.9.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Design Service Assurance (p. 826)	function	Canonical / Established
Process: Assurance Process Design (p. 827)	process	Canonical / Established
Feature: Assurance Disposition & Design Modelling (p. 828)	feature FT	Proposed / Proposed

26.9.9.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT1: Health Service Assurance (p. 202)	Established
To: enterprise contribution	Enterprise CT2: Managed Entity & Relationship (p. 838)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Context Management (p. 839)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Managed State & Lifecycle (p. 842)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Information Management (p. 844)	Established / Direct
To: enterprise contribution	Enterprise CT2: Search & Discovery (p. 849)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Policy & Control (p. 865)	Established / Direct
To: enterprise contribution	Enterprise CT2: Provenance & Traceability (p. 869)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Event & Subscription (p. 856)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Activity & Execution (p. 857)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Interaction & Experience (p. 862)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Operational Assurance (p. 873)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Service Guardian (p. 875)	Established / Direct

26.9.9.11. Function: Design Service Assurance

Property	Value	Meaning
Reference Id	`HCM-FN-design-service-assurance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN.FN-DESIGN-SERVICE-ASSURANCE`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification → BEC CT3: Assurance Design	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:71 (p. 1426)	Source standing does not approve the derived assertion.

26.9.9.11.1. Function Overview

Designing service assurance turns a requirement's assurance question into an explicit disposition and evaluation basis. The modeller determines how satisfaction is to be assessed, not what the authoritative requirement should mean or who may approve it. Design completion is not evidence that the implemented behaviour satisfies that requirement.

26.9.9.11.2. Definition / governed semantic detail

- **Canonical Name**: `DesignServiceAssurance`.
- **Definition**: Defines how satisfaction of a governed requirement, constraint or expected behaviour is to be independently established, including the applicable assurance disposition, assurance criteria, required evidence and evaluation expectations.
- **Boundary**: Concerns assurance design. It does not establish the underlying governance requirement, grant authority to it, perform the resulting assurance, manage the subject activity or prescribe application or implementation mechanisms. Modelling the resulting Assurance Definition does not grant approval authority. The Strategy peer relationship to behaviour design and failure/recovery design creates no additional Capability or Function.

26.9.9.11.3. Preserved v0.1 Review Matters

No contextual-view placement is established.

26.9.9.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: participating function	Process: Assurance Process Design (p. 827)	Established
To: realises feature	Feature: Assurance Disposition & Design Modelling (p. 828)	Proposed

26.9.9.12. Process: Assurance Process Design

Property	Value	Meaning
Reference Id	`HCM-PR-assurance-process-design`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN.PR-ASSURANCE-PROCESS-DESIGN`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification → BEC CT3: Assurance Design	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:355 (p. 1422); processes:358 (p. 1422)	Source standing does not approve the derived assertion.

26.9.9.12.1. Process Overview

Assurance design establishes a reusable basis for how satisfaction will be evaluated, rather than running the evaluation itself. Criteria-management participation retains its separate owner and does not create an exchange Service or a shared criteria lifecycle. An Assurance Definition can be reused, so each execution need not imply a freshly performed Design Process.

26.9.9.12.2. Definition / governed semantic detail

- **Canonical Name**: `AssuranceProcessDesign`.
- **Owning Capability**: Assurance Design.
- **Process Purpose**: Establishes and maintains how satisfaction of a governed requirement, constraint or expected behaviour is to be assured.
- **Principal Performing Role**: Service Assurance Modeller.
- **Participating Functions**: Design Service Assurance and Manage Assurance Criteria. Manage Assurance Criteria retains ownership within Assurance Criteria Management; its participation does not subsume that Capability's criteria lifecycle or transfer its information responsibility to Assurance Design.
- **Conceptual Output**: Assurance Definition, retaining its conceptual Business Architecture output/boundary treatment.
- **Boundary**: Governance Authority establishes what is authoritative. Design does not grant Service Assurance Modeller approval authority. Detailed Assurance Definition lifecycle, approval/publication workflows, criteria lifecycle and information modelling remain unresolved. Participation of the two Functions does not establish a cross-capability lifecycle or a new exchange/Service between their owners.

26.9.9.12.3. Preserved v0.1 Review Matters

No contextual-view placement is established.

26.9.9.12.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
To: participating function	Function: Design Service Assurance (p. 826)	Established
To: participating function	Function: Manage Assurance Criteria (p. 830)	Established

26.9.9.13. Feature: Assurance Disposition & Design Modelling

Property	Value	Meaning
Reference Id	`HCM-FT-assurance-disposition-design-modelling`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN.FEAT-STEP2-ASSURANCE-DISPOSITION-DESIGN-MODELLING`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification → BEC CT3: Assurance Design	Linked responsibility references; historical records do not assert current ancestry.
Evidence	health-service-assurance.md:9	Source standing does not approve the derived assertion.

26.9.9.13.1. Feature Overview

This proposal provides a Feature frame for the established design responsibility, including disposition, criteria and evidence expectations, without creating an additional assurance Function.

26.9.9.13.2. Definition — Proposed

Model how satisfaction of an applicable governed concern will be independently established.

26.9.9.13.3. Boundary

Modelling is distinct from requirement approval, subject management and evaluation.

26.9.9.13.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Assurance Design, rather than inheriting an adjacent responsibility.

26.9.9.13.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task health-service-assurance.md:9

26.9.9.13.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Design Service Assurance (p. 826)	Proposed

26.9.10. BEC CT3: Assurance Criteria Administration

Property	Value	Meaning
Reference Id	`HCM-BEC-assurance-criteria-management`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-CRITERIA-MANAGEMENT`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification	Linked responsibility references; historical records do not assert current ancestry.
Established name	Assurance Criteria Management	Current display rename is Proposed; meaning unchanged.
Name history	Assurance Criteria Management	Names do not establish another identity.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:686 (p. 1382)	Source standing does not approve the derived assertion.

26.9.10.1. Capability Overview

Administer reusable governed assurance criteria and their version-specific and temporal applicability without changing governing requirement authority.

26.9.10.2. Preserved Canonical semantic evidence — source context

The capability to establish and manage reusable, governed and temporally identifiable assurance criteria through which subjects may be evaluated.

26.9.10.3. Definition — Reviewed candidate

Administer reusable governed assurance criteria and their version-specific and temporal applicability without changing governing requirement authority.

26.9.10.4. Boundary — retained v0.1 source explanation

Modelling criteria does not inherently approve them. Detailed criterion states and approval workflows remain unresolved, and criteria need not all originate in formal Governance.

26.9.10.5. Harmonia Architectural Relevance — retained v0.1 source explanation

Formal relevance remains Unresolved. Established assurance support explains the enabling purpose without classifying it by inference or allocating software responsibility.

26.9.10.6. Boundary

Information administration does not confer authority over the represented person, organisation, service, place, device or clinical activity.

26.9.10.7. Proposed Placement Rationale

Responsibility-preserving placement under the human-reviewed BEC hierarchy; the structural assertion remains Proposed.

26.9.10.8. Reviewed reconciliation provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

26.9.10.9. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Function: Manage Assurance Criteria (p. 830)	function	Canonical / Established
Feature: Temporal Assurance Criteria Stewardship (p. 831)	feature FT	Proposed / Proposed

26.9.10.10. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enabled by	BC CT1: Health Service Assurance (p. 202)	Established
To: enterprise contribution	Enterprise CT2: Context Management (p. 839)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Managed State & Lifecycle (p. 842)	Established / Direct
To: enterprise contribution	Enterprise CT2: Information Management (p. 844)	Established / Direct
To: enterprise contribution	Enterprise CT2: Search & Discovery (p. 849)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Policy & Control (p. 865)	Established / Direct
To: enterprise contribution	Enterprise CT2: Provenance & Traceability (p. 869)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Interoperability & Exchange (p. 854)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Interaction & Experience (p. 862)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Operational Assurance (p. 873)	Established / Supporting
To: enterprise contribution	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Established / Direct
To: enterprise contribution	Enterprise CT2: Service Guardian (p. 875)	Established / Direct

26.9.10.11. Function: Manage Assurance Criteria

Property	Value	Meaning
Reference Id	`HCM-FN-manage-assurance-criteria`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-CRITERIA-MANAGEMENT.FN-MANAGE-ASSURANCE-CRITERIA`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification → BEC CT3: Assurance Criteria Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:84 (p. 1427)	Source standing does not approve the derived assertion.

26.9.10.11.1. Function Overview

Criteria management maintains the reusable evaluative basis and its temporal standing. It permits an assessment to be understood against the criterion applicable at the relevant time, rather than today's unqualified version. Management of that basis does not inherently grant approval authority or permit alteration of the governing obligation.

26.9.10.11.2. Definition / governed semantic detail

- **Canonical Name**: `ManageAssuranceCriteria`.
- **Definition**: Establishes and manages reusable, governed and temporally identifiable assurance criteria through which governed subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These examples are not an exhaustive taxonomy. Criteria may have lifecycle, version and temporal applicability.
- **Boundary**: Managing a criterion does not transfer authority for the governing requirement from which it may derive. Criteria may operationalise a requirement without silently altering, weakening, strengthening or replacing it; not every criterion must derive from formal Governance. Modelling/managing criteria does not inherently confer approval authority. Detailed criterion lifecycle states and approval workflows remain unresolved.

26.9.10.11.3. Preserved v0.1 Review Matters

No contextual-view placement is established.

26.9.10.11.4. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: participating function	Process: Assurance Process Design (p. 827)	Established
To: realises feature	Feature: Temporal Assurance Criteria Stewardship (p. 831)	Proposed

26.9.10.12. Feature: Temporal Assurance Criteria Stewardship

Property	Value	Meaning
Reference Id	`HCM-FT-temporal-assurance-criteria-stewardship`	Stable concept identity.

Property	Value	Meaning
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-CRITERIA-MANAGEMENT.FEAT-STEP2-TEMPORAL-ASSURANCE-CRITERIA-STEWARDSHIP`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Specification → BEC CT3: Assurance Criteria Administration	Linked responsibility references; historical records do not assert current ancestry.
Evidence	health-service-assurance.md:9	Source standing does not approve the derived assertion.

26.9.10.12.1. Feature Overview

Evidence must be evaluated against the criteria applicable to its purpose and time rather than an unqualified current rule.

26.9.10.12.2. Definition — Proposed

Manage identifiable assurance criteria and their relevant temporal and version applicability.

26.9.10.12.3. Boundary

Criteria stewardship does not approve governing requirements or select a universal result taxonomy.

26.9.10.12.4. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Assurance Criteria Management, rather than inheriting an adjacent responsibility.

26.9.10.12.5. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task health-service-assurance.md:9

26.9.10.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: realises feature	Function: Manage Assurance Criteria (p. 830)	Proposed

26.10. BEC CT2: Assurance Service Review

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-assurance-service-review`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SERVICE-REVIEW`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

26.10.1. Capability Overview

Independently review the operation, effectiveness, appropriateness and conformance of the assurance service itself. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

26.10.2. Definition — Reviewed candidate

Independently review the operation, effectiveness, appropriateness and conformance of the assurance service itself.

26.10.3. Intent

Independently review the operation, effectiveness, appropriateness and conformance of the assurance service itself.

26.10.4. Boundary

Reviews the assurance service itself. Canonical evidence association, subject assessment, adjudication, findings and conclusions are retained separately pending adjudication.

26.10.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

26.10.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
BEC CT3: Assurance Service Evaluation (p. 834)	capability CT3	Proposed / Proposed

26.10.7. BEC CT3: Assurance Service Evaluation

Property	Value	Meaning
Reference Id	`HCM-BEC-reviewed-assurance-service-evaluation`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SERVICE-REVIEW.CT3-BEC-ASSURANCE-SERVICE-EVALUATION`	Current proposed context where marked Proposed.
Current context	BEC CT1: Independent Assurance Enablement → BEC CT2: Assurance Service Review	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	reviewed-bec-hierarchy.json:2	Source standing does not approve the derived assertion.

26.10.7.1. Capability Overview

Evaluate the operation, effectiveness, appropriateness and conformance of the assurance service itself through independent review. The reviewed hierarchy assigns this enduring ability to its stated business-enabling context.

26.10.7.2. Definition — Reviewed candidate

Evaluate the operation, effectiveness, appropriateness and conformance of the assurance service itself through independent review.

26.10.7.3. Intent

Evaluate the operation, effectiveness, appropriateness and conformance of the assurance service itself through independent review.

26.10.7.4. Boundary

Evaluation concerns operation, effectiveness, appropriateness and conformance of the assurance service; it is not execution of assurance against a subject.

26.10.7.5. Derivation Provenance

Reconciliation against the explicit human-reviewed BEC candidate hierarchy. Bounded semantic reconciliation; no unconstrained synthesis reviewed-bec-hierarchy.json:2

Part IV.

Enterprise Capability Architecture

EC-01 through EC-14 retain Canonical responsibility, stable Reference ID and exact governed scope. Three Proposed CT1s and selected CT3 specialisations form one integrated hierarchy. Historical EC tokens are direct structural-ID aliases. No Feature is placed under an EC, and no application-component or deployment allocation is inferred.

CT1 / page	**Element status**	**Placement**
Enterprise CT1: Governed Information Operations (p. 837)	Proposed	Proposed
Enterprise CT1: Coordinated Activity & Interaction (p. 853)	Proposed	Proposed
Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Proposed	Proposed

27. Enterprise CT1: Governed Information Operations

Property	Value	Meaning
Reference Id	`HCM-EC-governed-information-operations`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS`	Current proposed context where marked Proposed.
Evidence	architectural-axioms.md:668; information-responsibility.md:10	Source standing does not approve the derived assertion.

27.1. Capability Overview

Entity integrity, context, lifecycle, information maintenance, discovery and semantic conformance are constituent reusable abilities; none is an implementation subsystem.

27.2. Definition — Proposed

Maintain identifiable, meaningful, context-qualified and retrievable information through valid state progression.

27.3. Intent

Keep information identity, meaning, context, state and retrieval compatible across diverse healthcare uses.

27.4. Boundary

Information management and semantic governance do not grant originating domain authority or erase active/durable state separation.

27.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:668; information-responsibility.md:10

27.6. Prior Candidate Derivation

v0.3 reconciliation: `EC-BBAD8E2C` (line 9792); `EC-47C59623` (line 10023); `EC-1F33C0E8` (line 10668). Working locators are not identity aliases.

27.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT2: Managed Entity & Relationship (p. 838)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Context Management (p. 839)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Managed State & Lifecycle (p. 842)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Information Management (p. 844)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Search & Discovery (p. 849)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Semantic Governance & Conformance (p. 851)	enterprise capability CT2	Canonical / Proposed

27.8. Enterprise CT2: Managed Entity & Relationship

Property	Value	Meaning
Reference Id	`EC-01`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-01`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-01`	Direct to this identity; never reusable.
Evidence	enterprise:103 (p. 1475)	Source standing does not approve the derived assertion.

27.8.1. Capability Overview

This reusable capability keeps managed healthcare entities identifiable and their relationships structurally intelligible. It supplies common identity, alias and relationship behaviour recurring in persons, practitioners, organisations, locations and devices. Reuse concerns those architectural responsibilities, allowing the healthcare-specific meaning of each entity to remain with its governing context.

27.8.2. Definition / governed semantic detail

- **Architectural Scope**: Identify, classify, relate, and maintain the structural integrity of managed healthcare and administrative entities throughout the enterprise landscape.
- **Functional Responsibilities**:
  - Uniquely identifying core domain entities (Persons, Practitioners, Roles, Organisations, Locations, Services, Devices).
  - Resolving and maintaining multi-authority identifier systems and aliases without destructive overwriting.
  - Representing and traversing multi-dimensional relationships (affiliations, organizational hierarchies, physical location trees, care team memberships).
  - Maintaining referential integrity across interconnected entity graphs.
- **Anti-Responsibilities**: Does not own physical database relational tables or foreign key constraints (Domain 04/07).

27.8.3. Boundary — v0.1 explanation

Relationship integrity is not a physical database design. Correlating entities does not establish originating clinical identity authority or a new standalone Correlation capability.

27.8.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.

27.8.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-C0A97A83` (line 9803); `EC-461B8F05` (line 9813); `EC-BAC316E4` (line 9913); `EC-F50F53E6` (line 9923). Working locators are not identity aliases.

27.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Person Identity Administration (p. 214)	Proposed
From: enterprise enabled by	BEC CT3: Healthcare Subject Profile Administration (p. 226)	Proposed
From: enterprise enabled by	BEC CT2: Provider Administration (p. 243)	Proposed
From: enterprise enabled by	BEC CT2: Organisation Administration (p. 263)	Proposed
From: enterprise enabled by	BEC CT2: Location Administration (p. 276)	Proposed
From: enterprise enabled by	BEC CT2: Health Service Administration (p. 287)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Device Administration (p. 305)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting

27.9. Enterprise CT2: Context Management

Property	Value	Meaning
Reference Id	`EC-02`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-02`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-02`	Direct to this identity; never reusable.
Evidence	enterprise:112 (p. 1475)	Source standing does not approve the derived assertion.

27.9.1. Capability Overview

Context Management preserves the circumstances in which information and activity have meaning as they cross processing boundaries. Subject, encounter, provider, service and security context must remain attributable and consistent rather than be reconstructed from convenient local assumptions. Its reuse is especially significant in multi-hop activity where a syntactically valid message can otherwise lose the context required for legitimate interpretation.

27.9.2. Definition / governed semantic detail

- **Architectural Scope**: Establish, validate, propagate, and preserve the execution, clinical, security, and administrative context within which information and activities have meaning.
- **Functional Responsibilities**:
  - Binding the subject of care, encounter, ordering provider, and healthcare service context to in-flight messages and transactions.
  - Propagating tamper-evident, attributable security context across asynchronous, multi-hop operational boundaries.
  - Ensuring context preservation during cross-protocol transformations across disparate healthcare exchange formats and protocols.
  - Detecting and rejecting requests where mandatory operational or clinical context is missing or inconsistent.
- **Collaborative Nature**: Bounded reusable context responsibility may be realised through participation across gateways, activity execution and information-access boundaries. Participation does not transfer semantic ownership.

27.9.3. Boundary

Context participation does not transfer semantic ownership. Generic context management and assurance-specific context semantics contribute together without allocating a new central Context Service.

27.9.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Governed Information Opera-

tions; its canonical scope and exclusions remain unchanged.

27.9.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-B80F9118` (line 10266); `EC-ADB90088` (line 10277); `EC-6F46C125` (line 10287). Working locators are not identity aliases.

27.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Client Privacy Guardrail Management (p. 237)	Proposed
From: enterprise enabled by	BEC CT2: Health Information Control (p. 715)	Proposed
From: enterprise enabled by	BEC CT2: Referral Administration (p. 342)	Proposed
From: enterprise enabled by	BEC CT2: Order Administration (p. 383)	Proposed
From: enterprise enabled by	BEC CT2: Diagnostic Administration (p. 399)	Proposed
From: enterprise enabled by	BEC CT2: Medication Administration (p. 417)	Proposed
From: enterprise enabled by	BEC CT2: Procedure Administration (p. 435)	Proposed
From: enterprise enabled by	BEC CT2: Episode & Encounter Administration (p. 363)	Proposed
From: enterprise enabled by	BEC CT3: Primary Care Coordination (p. 669)	Proposed
From: enterprise enabled by	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Proposed
From: enterprise enabled by	BEC CT3: Emergency Care Progression (p. 554)	Proposed
From: enterprise enabled by	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Proposed
From: enterprise enabled by	BEC CT3: Community Care Coordination (p. 460)	Proposed
From: enterprise enabled by	BEC CT3: Outreach Care Continuity (p. 466)	Proposed
From: enterprise enabled by	BEC CT3: Remote Patient Monitoring (p. 675)	Proposed
From: enterprise enabled by	BEC CT3: Screening and Immunisation Enablement (p. 329)	Proposed
From: enterprise enabled by	BEC CT3: Diagnostic Service Integration (p. 401)	Proposed
From: enterprise enabled by	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Proposed
From: enterprise enabled by	BEC CT2: Clinic & Practice Operations (p. 514)	Proposed
From: enterprise enabled by	BEC CT2: Ward Operations (p. 524)	Proposed
From: enterprise enabled by	BEC CT2: Theatre Operations (p. 538)	Proposed
From: enterprise enabled by	BEC CT2: Emergency Department Operations (p. 548)	Proposed
From: enterprise enabled by	BEC CT2: Outpatient Operations (p. 561)	Proposed
From: enterprise enabled by	BEC CT2: Mobile Staff Management (p. 591)	Proposed
From: enterprise enabled by	BEC CT2: Service Capacity Management (p. 581)	Proposed

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Work Allocation & Dispatch (p. 607)	Proposed
From: enterprise enabled by	BEC CT2: Patient Transport (p. 632)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Logistics Coordination (p. 639)	Proposed
From: enterprise enabled by	BEC CT2: Discharge Management (p. 645)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Supporting

27.10. Enterprise CT2: Managed State & Lifecycle

Property	Value	Meaning
Reference Id	`EC-03`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-03`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-03`	Direct to this identity; never reusable.
Evidence	enterprise:121 (p. 1476)	Source standing does not approve the derived assertion.

27.10.1. Capability Overview

Managed State & Lifecycle provides reusable responsibility for valid progression, temporal applicability and non-destructive change. A directory relationship, clinical document and operational work item have different business states, yet all need an intelligible distinction between current standing, historical state and invalid transitions. The capability captures that recurrence without replacing each subject's lifecycle meaning.

27.10.2. Definition / governed semantic detail

- **Architectural Scope**: Govern the current and historical state, effective temporal periods, state transition validations, and lifecycle progression of managed integration artifacts and entities.
- **Functional Responsibilities**:
  - Defining and enforcing valid state transition state machines (e.g., `REQUESTED` → `VALIDATED` → `IN_PROGRESS` → `COMPLETED`).

  - Maintaining temporal validity intervals (effective start and end dates) for relationships, credentials, and states.
  - Supporting non-destructive state progression, preserving historical state versions for longitudinal review.
  - Rejecting illegal or out-of-order state transitions while accommodating idempotent replay.
- **Anti-Responsibilities**: Does not define thread-level mutexes or database transaction isolation levels.

27.10.3. Boundary

Business lifecycle integrity does not prescribe locks, database isolation or worker implementation, and illustrative state names do not define every subject's transitions.

27.10.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.

27.10.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-260111DC` (line 10035); `EC-AE5E9793` (line 10045); `EC-ADB90088` (line 10277); `EC-6F46C125` (line 10287). Working locators are not identity aliases.

27.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Person Identity Administration (p. 214)	Proposed
From: enterprise enabled by	BEC CT3: Healthcare Subject Profile Administration (p. 226)	Proposed
From: enterprise enabled by	BEC CT2: Provider Administration (p. 243)	Proposed
From: enterprise enabled by	BEC CT2: Organisation Administration (p. 263)	Proposed
From: enterprise enabled by	BEC CT2: Location Administration (p. 276)	Proposed
From: enterprise enabled by	BEC CT2: Health Service Administration (p. 287)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Device Administration (p. 305)	Proposed
From: enterprise enabled by	BEC CT2: Referral Administration (p. 342)	Proposed
From: enterprise enabled by	BEC CT2: Order Administration (p. 383)	Proposed
From: enterprise enabled by	BEC CT2: Diagnostic Administration (p. 399)	Proposed
From: enterprise enabled by	BEC CT2: Medication Administration (p. 417)	Proposed
From: enterprise enabled by	BEC CT2: Procedure Administration (p. 435)	Proposed
From: enterprise enabled by	BEC CT2: Episode & Encounter Administration (p. 363)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Record Administration (p. 443)	Proposed
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
From: enterprise enabled by	BEC CT2: Care Plan Administration (p. 455)	Proposed

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Clinic & Practice Operations (p. 514)	Proposed
From: enterprise enabled by	BEC CT2: Ward Operations (p. 524)	Proposed
From: enterprise enabled by	BEC CT2: Theatre Operations (p. 538)	Proposed
From: enterprise enabled by	BEC CT2: Emergency Department Operations (p. 548)	Proposed
From: enterprise enabled by	BEC CT2: Outpatient Operations (p. 561)	Proposed
From: enterprise enabled by	BEC CT2: Mobile Staff Management (p. 591)	Proposed
From: enterprise enabled by	BEC CT2: Service Capacity Management (p. 581)	Proposed
From: enterprise enabled by	BEC CT2: Information Design Governance (p. 792)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed
From: enterprise enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Direct

27.11. Enterprise CT2: Information Management

Property	Value	Meaning
Reference Id	`EC-04`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-04`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-04`	Direct to this identity; never reusable.
Evidence	enterprise:130 (p. 1476)	Source standing does not approve the derived assertion.

27.11.1. Capability Overview

Information Management provides the recurring ability to represent and maintain governed information through validation, preservation, retrieval and versioned change. It accommodates

both responsive active use and authoritative durable preservation while keeping those responsibilities separate. This distinction allows availability and working state to support care without turning a cache into durable truth.

27.11.2. Definition / governed semantic detail

- **Architectural Scope**: Validate, represent, persist, retrieve, version, and manage governed clinical, administrative, and operational information assets.
- **Functional Responsibilities**:
  - Validating inbound information structures against canonical information constraints.
  - Providing authoritative durable persistence for historical health records and administrative registries.
  - Managing active distributed working memory representations for low-latency retrieval.
  - Governing document versioning, supersession, amendments, and retractions without data loss.
- **Architectural Principle**: Enforces the foundational separation between *active distributed state access* and *authoritative durable state preservation* (Axiom AX-05).

27.11.3. Boundary — v0.1 explanation

Engine capabilities provide machinery under Harmonia semantics. Persistence success does not establish information authority or make the platform the originator of the represented facts.

27.11.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.

27.11.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-260111DC` (line 10035); `EC-AE5E9793` (line 10045). Working locators are not identity aliases.

27.11.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT3: Active Information Availability (p. 846)	enterprise capability CT3	Proposed / Proposed
Enterprise CT3: Authoritative Durable State Establishment (p. 847)	enterprise capability CT3	Proposed / Proposed
Enterprise CT3: Binary Content Association & Resolution (p. 848)	enterprise capability CT3	Proposed / Proposed

27.11.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Clinical Record Administration (p. 443)	Proposed
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
From: enterprise enabled by	BEC CT2: Care Plan Administration (p. 455)	Proposed
From: enterprise enabled by	BEC CT3: Primary Care Coordination (p. 669)	Proposed
From: enterprise enabled by	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Proposed

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Emergency Care Progression (p. 554)	Proposed
From: enterprise enabled by	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Proposed
From: enterprise enabled by	BEC CT3: Community Care Coordination (p. 460)	Proposed
From: enterprise enabled by	BEC CT3: Outreach Care Continuity (p. 466)	Proposed
From: enterprise enabled by	BEC CT3: Remote Patient Monitoring (p. 675)	Proposed
From: enterprise enabled by	BEC CT3: Screening and Immunisation Enablement (p. 329)	Proposed
From: enterprise enabled by	BEC CT3: Diagnostic Service Integration (p. 401)	Proposed
From: enterprise enabled by	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Direct
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Direct

27.11.8. Enterprise CT3: Active Information Availability

Property	Value	Meaning
Reference Id	`HCM-EC-active-information-availability`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-04.CT3-EC-ACTIVE-INFORMATION-AVAILABILITY`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations → Enterprise CT2: Information Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:247; context-and-persistence-management.md:52	Source standing does not approve the derived assertion.

27.11.8.1. Capability Overview

Active representation, observation and convergence support availability while preserving an explicit trust basis for use.

27.11.8.2. Definition — Proposed

Maintain responsive, reconstructable active information and coordinate its safe use.

27.11.8.3. Intent

Provide responsive managed use without promoting active representation into durable truth.

27.11.8.4. Boundary

An active generation is not an authoritative version; failure to converge does not establish valid active state.

27.11.8.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:247; context-and-persistence-management.md:52

27.11.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
From: enterprise enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed

27.11.9. Enterprise CT3: Authoritative Durable State Establishment

Property	Value	Meaning
Reference Id	`HCM-EC-authoritative-durable-state-establishment`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-04.CT3-EC-AUTHORITATIVE-DURABLE-STATE-ESTABLISHMENT`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations → Enterprise CT2: Information Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:247	Source standing does not approve the derived assertion.

27.11.9.1. Capability Overview

Durable acceptance gives managed information an explicit committed state from which applicable active representations can be reconstructed.

27.11.9.2. Definition — Proposed

Atomically establish durable managed state and its authoritative version progression.

27.11.9.3. Intent

Give managed change an explicit committed state and authoritative progression basis.

27.11.9.4. Boundary

Commit is distinct from semantic source authority and from successful active convergence; applications do not gain a raw persistence path.

27.11.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:247

27.11.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
From: enterprise enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed

27.11.10. Enterprise CT3: Binary Content Association & Resolution

Property	Value	Meaning
Reference Id	`HCM-EC-binary-content-association-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-04.CT3-EC-BINARY-CONTENT-ASSOCIATION-RESOLUTION`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations → Enterprise CT2: Information Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	sylloge-binary-content-management.md:14; information-responsibility.md:10	Source standing does not approve the derived assertion.

27.11.10.1. Capability Overview

Document attachments, diagnostic media and operational artefacts share a need to locate usable content while distinguishing logical meaning from physical possession.

27.11.10.2. Definition — Proposed

Maintain content associations and custody knowledge sufficient for governed payload resolution, retrieval and assembly.

27.11.10.3. Intent

Make logical information and its usable payloads consistently associable while retaining custody boundaries.

27.11.10.4. Boundary

No universal binary custody, storage topology or persistence authority is implied.

27.11.10.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task sylloge-binary-content-management.md:14; information-responsibility.md:10

27.11.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Linked Information Curation (p. 663)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Record Administration (p. 443)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Collaboration (p. 756)	Proposed

27.12. Enterprise CT2: Search & Discovery

Property	Value	Meaning
Reference Id	`EC-05`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-05`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-05`	Direct to this identity; never reusable.

Property	Value	Meaning
Evidence	enterprise:139 (p. 1476)	Source standing does not approve the derived assertion.

27.12.1. Capability Overview

Search & Discovery makes information, entities and service endpoints findable through governed criteria and bounded queries. It addresses the recurring need to locate relevant material or a destination before it can be used. Security-qualified results preserve the distinction between knowing that something exists and being entitled to obtain or disclose it.

27.12.2. Definition / governed semantic detail

- **Architectural Scope**: Index, search, resolve, filter, and discover information assets, entities, directory endpoints, and healthcare services across the enterprise.
- **Functional Responsibilities**:
  - Providing multi-criteria indexed search across complex entity graphs (e.g., find active cardiologists at Location X accepting new referrals).
  - Resolving service and practitioner communication endpoints for addressed delivery.
  - Supporting bounded, paginated clinical queries across longitudinal patient records.
  - Applying dynamic security and consent filters to search results prior to egress.
- **Anti-Responsibilities**: Does not dictate specific search engine indexes, inverted index implementations, or SQL query syntaxes.

27.12.3. Boundary

Discovery does not decide policy or prescribe a search engine, index format or query language.

27.12.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.

27.12.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-2A3844C6` (line 10127); `EC-299D0172` (line 10137). Working locators are not identity aliases.

27.12.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Person Identity Administration (p. 214)	Proposed
From: enterprise enabled by	BEC CT3: Healthcare Subject Profile Administration (p. 226)	Proposed
From: enterprise enabled by	BEC CT2: Provider Administration (p. 243)	Proposed
From: enterprise enabled by	BEC CT2: Organisation Administration (p. 263)	Proposed
From: enterprise enabled by	BEC CT2: Location Administration (p. 276)	Proposed
From: enterprise enabled by	BEC CT2: Health Service Administration (p. 287)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Device Administration (p. 305)	Proposed
From: enterprise enabled by	BEC CT2: Health Information Access (p. 701)	Proposed

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed
From: enterprise enabled by	BEC CT2: On-Call Management (p. 601)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Supporting

27.13. Enterprise CT2: Semantic Governance & Conformance

Property	Value	Meaning
Reference Id	`EC-13`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-13`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Governed Information Operations	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-13`	Direct to this identity; never reusable.
Evidence	enterprise:215 (p. 1479)	Source standing does not approve the derived assertion.

27.13.1. Capability Overview

This capability makes canonical information models, terminology bindings, mappings and conformance rules governable across enterprise use. It addresses both what representations mean and whether they respect the published rules, keeping version and compatibility concerns explicit. A conformant representation can therefore be distinguished from one that merely uses a supported transport or parser.

27.13.2. Definition / governed semantic detail

- **Architectural Scope**: Govern canonical models, schemas, terminologies, value sets, mapping tables, constraint rules, and semantic conformance validation across the enterprise.
- **Functional Responsibilities**:
  - Defining and publishing canonical healthcare data models and profile structures.
  - Governing clinical terminology mappings, value sets, and concept lookups (SNOMED CT, LOINC).
  - Validating inbound and outbound message representations against published semantic rules.
  - Governing semantic versioning, deprecation, and backward compatibility across schema revisions.

- **Component Relationship**: Informs the Calliope strategic logical component, but Calliope is the responsibility centre, not the capability definition.

27.13.3. Boundary — v0.1 explanation

Conformance does not establish clinical truth, information authority or disclosure permission. Calliope is an established responsibility centre, not the definition of the capability.

27.13.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.

27.13.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-89E071D0` (line 10210); `EC-47BECE6F` (line 10220). Working locators are not identity aliases.

27.13.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Information Design Governance (p. 792)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed
From: enterprise enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Direct

28. Enterprise CT1: Coordinated Activity & Interaction

Property	Value	Meaning
Reference Id	`HCM-EC-coordinated-activity-interaction`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION`	Current proposed context where marked Proposed.
Evidence	architectural-axioms.md:668; information-responsibility.md:10	Source standing does not approve the derived assertion.

28.1. Capability Overview

Requests, events, work and human interaction are related ways to cause or observe change, with distinct contracts and progression responsibilities.

28.2. Definition — Proposed

Enable governed interaction, event participation, boundary exchange and accountable activity progression.

28.3. Intent

Make requests, event participation, work and human interaction progress according to their different contracts.

28.4. Boundary

Exchange is not execution, notification is not outcome, and interaction is not clinical authority.

28.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:668; information-responsibility.md:10

28.6. Prior Candidate Derivation

v0.3 reconciliation: `EC-B80F9118` (line 10266); `EC-1F33C0E8` (line 10668). Working locators are not identity aliases.

28.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT2: Interoperability & Exchange (p. 854)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Event & Subscription (p. 856)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Activity & Execution (p. 857)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Interaction & Experience (p. 862)	enterprise capability CT2	Canonical / Proposed

28.8. Enterprise CT2: Interoperability & Exchange

Property	Value	Meaning
Reference Id	`EC-08`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-08`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-08`	Direct to this identity; never reusable.
Evidence	enterprise:166 (p. 1477)	Source standing does not approve the derived assertion.

28.8.1. Capability Overview

Interoperability & Exchange moves information across participating boundaries through destination resolution, transformation, delivery and acknowledgements. It preserves the difference between technical transmission and the recipient's business response so that an exchange can be understood under its actual contract. Standards govern the external representation while internal management semantics remain private.

28.8.2. Definition / governed semantic detail

- **Architectural Scope**: Resolve destinations, transform schemas, route, transport, deliver, acknowledge, and coordinate standards-based information exchange across enterprise boundaries.
- **Functional Responsibilities**:
  - Providing multi-protocol boundary adaptation for healthcare standards (HL7 v2, FHIR REST, DICOM, secure messaging).
  - Executing bidirectional structural and semantic transformations between legacy formats and canonical representations.
  - Ensuring reliable addressed delivery and fan-out distribution to external clinical endpoints.
  - Processing and distinguishing technical delivery acknowledgements from business acknowledgements.
- **Boundary Rule**: Egress terminates Harmonia management of the emitted representation (Axiom AX-13).

28.8.3. Boundary

Egress ends Harmonia management of the emitted representation. Retained delivery evidence does not extend control over an external recipient or transfer payload authority.

28.8.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.

28.8.5. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Referral Administration (p. 342)	Proposed
From: enterprise enabled by	BEC CT2: Order Administration (p. 383)	Proposed
From: enterprise enabled by	BEC CT2: Diagnostic Administration (p. 399)	Proposed
From: enterprise enabled by	BEC CT2: Medication Administration (p. 417)	Proposed
From: enterprise enabled by	BEC CT2: Procedure Administration (p. 435)	Proposed
From: enterprise enabled by	BEC CT2: Episode & Encounter Administration (p. 363)	Proposed
From: enterprise enabled by	BEC CT2: Health Information Exchange Enablement (p. 686)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Communication Administration (p. 476)	Proposed
From: enterprise enabled by	BEC CT2: Population Health Information Reporting (p. 817)	Proposed
From: enterprise enabled by	BEC CT3: Primary Care Coordination (p. 669)	Proposed
From: enterprise enabled by	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Proposed
From: enterprise enabled by	BEC CT3: Emergency Care Progression (p. 554)	Proposed
From: enterprise enabled by	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Proposed
From: enterprise enabled by	BEC CT3: Community Care Coordination (p. 460)	Proposed

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Outreach Care Continuity (p. 466)	Proposed
From: enterprise enabled by	BEC CT3: Remote Patient Monitoring (p. 675)	Proposed
From: enterprise enabled by	BEC CT3: Screening and Immunisation Enablement (p. 329)	Proposed
From: enterprise enabled by	BEC CT3: Diagnostic Service Integration (p. 401)	Proposed
From: enterprise enabled by	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Proposed
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Supporting

28.9. Enterprise CT2: Event & Subscription

Property	Value	Meaning
Reference Id	`EC-09`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-09`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-09`	Direct to this identity; never reusable.
Evidence	enterprise:175 (p. 1478)	Source standing does not approve the derived assertion.

28.9.1. Capability Overview

Event & Subscription connects significant changes with participants whose registered interests match them. It makes event ingestion, correlation, interest criteria and notification reusable across clinical and operational contexts. The architectural meaning is the relationship between a change and an interested consumer, rather than the broker constructs used to carry it.

28.9.2. Definition / governed semantic detail

- **Architectural Scope**: Publish, ingest, correlate, filter, and syndicate events and subscription-driven changes across the enterprise landscape.
- **Functional Responsibilities**:
  - Ingesting operational and clinical event triggers from external systems and internal state changes.
  - Managing topic- and criteria-based event subscriptions for internal and external consumers.

  - Evaluating dynamic subscription filters to match event payloads with interested subscriber criteria.
  - Delivering asynchronous event notifications reliably to registered subscribers.
- **Decoupling Note**: Defined strictly in terms of event publishing, criteria matching, and notification delivery; entirely independent of broker topics, partitions, or queues.

28.9.3. Boundary

Receiving a notification does not determine the consumer's business response; topics and queues do not define the capability.

28.9.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.

28.9.5. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Health Information Exchange Enablement (p. 686)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Communication Administration (p. 476)	Proposed
From: enterprise enabled by	BEC CT2: Population Health Information Reporting (p. 817)	Proposed
From: enterprise enabled by	BEC CT2: Clinic & Practice Operations (p. 514)	Proposed
From: enterprise enabled by	BEC CT2: Ward Operations (p. 524)	Proposed
From: enterprise enabled by	BEC CT2: Theatre Operations (p. 538)	Proposed
From: enterprise enabled by	BEC CT2: Emergency Department Operations (p. 548)	Proposed
From: enterprise enabled by	BEC CT2: Outpatient Operations (p. 561)	Proposed
From: enterprise enabled by	BEC CT2: Mobile Staff Management (p. 591)	Proposed
From: enterprise enabled by	BEC CT2: Service Capacity Management (p. 581)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Collaboration (p. 756)	Proposed
From: enterprise enabled by	BEC CT2: General Collaboration (p. 768)	Proposed
From: enterprise enabled by	BEC CT2: Presentation Services (p. 786)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting

28.10. Enterprise CT2: Activity & Execution

Property	Value	Meaning
Reference Id	`EC-10`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.

Property	Value	Meaning
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-10`	Direct to this identity; never reusable.
Evidence	enterprise:184 (p. 1478)	Source standing does not approve the derived assertion.

28.10.1. Capability Overview

Activity & Execution provides reusable instantiation, matching, dispatch and supervision for governed activity units. Human Work Orders, human decision To Dos and automated Synthetic Tasks retain different performer and authority semantics while sharing a need for accountable progression. This reuse supports operational coordination without making a runtime engine the definition of the work.

28.10.2. Definition / governed semantic detail

- **Architectural Scope**: Define, instantiate, match, assign, dispatch, supervise, and progress coordinated operational and clinical activity units.
- **Functional Responsibilities**:
  - Instantiating operational work items (Work Orders, To Dos, synthetic platform Tasks) in response to triggers.
  - Matching and assigning work units to eligible human or automated performers and roles based on policy rules.
  - Dispatching work units to their assigned performers with acknowledgement according to the applicable interaction contract.
  - Supervising execution progress, tracking timeouts, managing escalations, and handling activity interruptions.
- **Decoupling Note**: Free of worker threads, thread pools, execution daemons, or specific workflow runtime engines.

28.10.3. Boundary

Coordination does not confer clinical work allocation or judgement. The capability specifies neither worker threads nor a particular execution engine.

28.10.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.

28.10.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-8C0E52A8` (line 10396); `EC-D05CE29D` (line 10406). Working locators are not identity aliases.

28.10.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT3: Operational Fulfilment Definition (p. 859)	enterprise capability CT3	Proposed / Proposed
Enterprise CT3: Governed Activity Execution Control (p. 860)	enterprise capability CT3	Proposed / Proposed
Enterprise CT3: Entity-Relative Operational Progression (p. 861)	enterprise capability CT3	Proposed / Proposed

28.10.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Referral Administration (p. 342)	Proposed
From: enterprise enabled by	BEC CT2: Order Administration (p. 383)	Proposed
From: enterprise enabled by	BEC CT2: Diagnostic Administration (p. 399)	Proposed
From: enterprise enabled by	BEC CT2: Medication Administration (p. 417)	Proposed
From: enterprise enabled by	BEC CT2: Procedure Administration (p. 435)	Proposed
From: enterprise enabled by	BEC CT2: Episode & Encounter Administration (p. 363)	Proposed
From: enterprise enabled by	BEC CT2: Work Allocation & Dispatch (p. 607)	Proposed
From: enterprise enabled by	BEC CT2: Patient Transport (p. 632)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Logistics Coordination (p. 639)	Proposed
From: enterprise enabled by	BEC CT2: Discharge Management (p. 645)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting

28.10.8. Enterprise CT3: Operational Fulfilment Definition

Property	Value	Meaning
Reference Id	`HCM-EC-operational-fulfilment-definition`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10.CT3-EC-OPERATIONAL-FULFILMENT-DEFINITION`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction → Enterprise CT2: Activity & Execution	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	context-and-persistence-management.md:52; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

28.10.8.1. Capability Overview

The semantic execution envelope applies to one-step and multi-step activity and stays distinct from dispatch and supervision.

28.10.8.2. Definition — Proposed

Define the context requirements, expected outcomes and housekeeping of an operational undertaking.

28.10.8.3. Intent

Keep the semantic promise of an undertaking separate from the machinery that executes it.

28.10.8.4. Boundary

A fulfilment definition does not execute, orchestrate or choreograph its activity.

28.10.8.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task context-and-persistence-management.md:52; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

28.10.9. Enterprise CT3: Governed Activity Execution Control

Property	Value	Meaning
Reference Id	`HCM-EC-governed-activity-execution-control`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10.CT3-EC-GOVERNED-ACTIVITY-EXECUTION-CONTROL`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction → Enterprise CT2: Activity & Execution	Linked responsibility references; historical records do not assert current ancestry.
Evidence	05-intrinsic-enablement.md:170; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

28.10.9.1. Capability Overview

Reusable execution control respects work-unit archetype, participation and uncertain effects rather than equating dispatch with completion.

28.10.9.2. Definition — Proposed

Instantiate, dispatch, supervise and progress authorised operational undertakings with explicit outcome standing.

28.10.9.3. Intent

Progress accountable operational work without equating dispatch, execution and successful outcome.

28.10.9.4. Boundary

Does not own underlying clinical-work meaning, clinical decisions or originator business authority.

28.10.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task 05-intrinsic-enablement.md:170; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

28.10.10. Enterprise CT3: Entity-Relative Operational Progression

Property	Value	Meaning
Reference Id	`HCM-EC-entity-relative-operational-progression`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10.CT3-EC-ENTITY-RELATIVE-OPERATIONAL-PROGRESSION`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction → Enterprise CT2: Activity & Execution	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

28.10.10.1. Capability Overview

Patient, practitioner and bed concerns recur across different business contexts; each can need a coherent progression frame without one central entity service.

28.10.10.2. Definition — Proposed

Coordinate serial, entity-relative operational inputs and their interested behaviours.

28.10.10.3. Intent

Maintain a coherent operational reference frame as one entity reacts to successive inputs.

28.10.10.4. Boundary

Interest fan-out is distinct from intra-entity concurrency; entity progression does not acquire originating facts.

28.10.10.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

28.10.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Discharge Management (p. 645)	Proposed
From: enterprise enabled by	BEC CT2: Bed & Care-Place Management (p. 569)	Proposed

28.11. Enterprise CT2: Interaction & Experience

Property	Value	Meaning
Reference Id	`EC-11`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-11`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Coordinated Activity & Interaction	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-11`	Direct to this identity; never reusable.
Evidence	enterprise:193 (p. 1478)	Source standing does not approve the derived assertion.

28.11.1. Capability Overview

Interaction & Experience makes contextual information and governed actions available to human participants through presentation and collaboration integration. Clinical timelines, directory administration and operational dashboards have different purposes, but share a need to retain established identity, security and information context. The recurring responsibility is usable interaction within those boundaries.

28.11.2. Definition / governed semantic detail

- **Architectural Scope**: Provide contextual interaction between human actors, Harmonia, and enterprise presentation services, including visualization, alerting, and collaboration integration.
- **Functional Responsibilities**:
  - Mediating presentation interactions via dedicated backend-for-frontend gateways decoupled from internal databases.
  - Rendering clinical timelines, provider directory management interfaces, and operational console dashboards.
  - Providing contextual notifications and alerts to human users across web and mobile channels.
  - Projecting longitudinal record context into secure multidisciplinary collaboration spaces.
- **Boundary Note**: Does not absorb the healthcare semantics of Clinical Collaboration or medical documentation. Consumes established identity and security context; does not own authentication, consent, or clinical truth.

28.11.3. Boundary

Presentation does not own consent, authentication or clinical truth, and collaboration projection does not absorb clinical documentation semantics.

28.11.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.

28.11.5. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Clinical Collaboration (p. 756)	Proposed
From: enterprise enabled by	BEC CT2: General Collaboration (p. 768)	Proposed
From: enterprise enabled by	BEC CT2: Presentation Services (p. 786)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Supporting

29. Enterprise CT1: Trust, Accountability & Assurance

Property	Value	Meaning
Reference Id	`HCM-EC-trust-accountability-assurance`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE`	Current proposed context where marked Proposed.
Evidence	architectural-axioms.md:668; information-responsibility.md:10	Source standing does not approve the derived assertion.

29.1. Capability Overview

Policy, evidence, resilience and independent assurance contribute to confidence through different responsibilities. Keeping them as peers prevents technical success or recorded evidence from becoming a business assurance conclusion.

29.2. Definition — Proposed

Make governed activity permissible, attributable, operationally dependable and independently assessable.

29.3. Intent

Keep permission, attribution, operational integrity and independent confidence distinguishable and usable together.

29.4. Boundary

Cross-cutting participation does not distribute semantic ownership, mandate centralisation or authorise the assured subject to determine its own conclusion.

29.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:668; information-responsibility.md:10

29.6. Prior Candidate Derivation

v0.3 reconciliation: `EC-8B8BC538` (line 10470); `EC-1F33C0E8` (line 10668). Working locators are not identity aliases.

29.7. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT2: Policy & Control (p. 865)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Provenance & Traceability (p. 869)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Operational Assurance (p. 873)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Service Guardian (p. 875)	enterprise capability CT2	Canonical / Proposed
Enterprise CT2: Information Authority Qualification (p. 877)	enterprise capability CT2	Proposed / Proposed

29.8. Enterprise CT2: Policy & Control

Property	Value	Meaning
Reference Id	`EC-06`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-06`	Direct to this identity; never reusable.
Evidence	enterprise:148 (p. 1477)	Source standing does not approve the derived assertion.

29.8.1. Capability Overview

Policy & Control supplies the reusable evaluation and enforcement of authority, consent, privacy and handling constraints. Its purpose is consistent governed action across ingress, information access, execution and presentation boundaries. Default-deny evaluation preserves the need for established authority rather than treating a caller's intent or a technically valid request as permission.

29.8.2. Definition / governed semantic detail

- **Architectural Scope**: Evaluate and enforce security, consent, privacy, and regulatory policies including authority, access control, and information handling rules.
- **Functional Responsibilities**:
  - Executing impartial, default-deny policy evaluations for all integration transactions.
  - Enforcing Role-Based and Attribute-Based Access Control (RBAC/ABAC) policies.
  - Evaluating patient consent directives and privacy restrictions at boundary and query checkpoints.
  - Enforcing statutory information disclosure boundaries (e.g., mental health, VIP records).
- **Collaborative Nature**: Evaluated across boundary gateways, internal bus dispatch, storage interfaces, and presentation layers.

29.8.3. Boundary

Participation at multiple enforcement points does not distribute policy ownership or turn operational security context into persistent clinical content.

29.8.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.

29.8.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-D9AF0B6F` (line 10482); `EC-E37E56AC` (line 10492). Working locators are not identity aliases.

29.8.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT3: Operation Policy Resolution (p. 867)	enterprise capability CT3	Proposed / Proposed
Enterprise CT3: Operation-Bound Policy Enforcement (p. 868)	enterprise capability CT3	Proposed / Proposed

29.8.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Client Privacy Guardrail Management (p. 237)	Proposed
From: enterprise enabled by	BEC CT2: Health Information Control (p. 715)	Proposed
From: enterprise enabled by	BEC CT2: Health Information Access (p. 701)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed
From: enterprise enabled by	BEC CT2: On-Call Management (p. 601)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Collaboration (p. 756)	Proposed
From: enterprise enabled by	BEC CT2: General Collaboration (p. 768)	Proposed
From: enterprise enabled by	BEC CT2: Presentation Services (p. 786)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Direct

Relationship	Related element	Assertion status
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Direct

29.8.8. Enterprise CT3: Operation Policy Resolution

Property	Value	Meaning
Reference Id	`HCM-EC-operation-policy-resolution`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06.CT3-EC-OPERATION-POLICY-RESOLUTION`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance → Enterprise CT2: Policy & Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33	Source standing does not approve the derived assertion.

29.8.8.1. Capability Overview

Permission depends on controlled criteria, role assignments, action and information scope; revocation is distinct from natural expiry.

29.8.8.2. Definition — Proposed

Resolve effective, temporally applicable permission criteria for an established identity and governed operation.

29.8.8.3. Intent

Determine the effective permitted action under the applicable identity, information and temporal criteria.

29.8.8.4. Boundary

Does not authenticate identity or grant permission from caller-controlled tags or mere role names.

29.8.8.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33

29.8.9. Enterprise CT3: Operation-Bound Policy Enforcement

Property	Value	Meaning
Reference Id	`HCM-EC-operation-bound-policy-enforcement`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06.CT3-EC-OPERATION-BOUND-POLICY-ENFORCEMENT`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance → Enterprise CT2: Policy & Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33; architectural-axioms.md:348	Source standing does not approve the derived assertion.

29.8.9.1. Capability Overview

Distributed participants apply the applicable effective authorisation and fail closed when authority is absent or untrustworthy.

29.8.9.2. Definition — Proposed

Enforce resolved policy constraints at the boundary where a governed action is attempted.

29.8.9.3. Intent

Ensure an operation cannot proceed merely because its caller omitted or bypassed a voluntary policy check.

29.8.9.4. Boundary

Enforcement participation does not distribute policy ownership or make security context persistent content.

29.8.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33; architectural-axioms.md:348

29.9. Enterprise CT2: Provenance & Traceability

Property	Value	Meaning
Reference Id	`EC-07`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-07`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-07`	Direct to this identity; never reusable.
Evidence	enterprise:157 (p. 1477)	Source standing does not approve the derived assertion.

29.9.1. Capability Overview

Provenance & Traceability preserves the meaningful attribution and history needed to explain information and significant activity. It connects source, transformation and required checkpoints without making every internal mechanism permanent evidence. Its recurring contribution supports accountability, reconstruction and assurance use while retaining the context that makes a recorded fact evidentially relevant.

29.9.2. Definition / governed semantic detail

- **Architectural Scope**: Preserve required origin attribution, meaningful transformation history, significant execution checkpoints, tamper-evident audit evidence and end-to-end traceability. Harmonia SHALL preserve provenance, audit and operational evidence where required by governing obligations, architectural significance, accountability, reconstruction, security or assurance needs. Routine technical activity SHALL NOT automatically require durable evidentiary recording.
- **Functional Responsibilities**:
  - Recording attributable, verifiable origin metadata for information sources, authors, and timestamps.
  - Tracking information-significant transformation history and required mapping checkpoints as information crosses system seams.
  - Capturing non-PHI operational and security audit trails for regulatory compliance (HIPAA, GDPR).
  - Supporting end-to-end transaction tracing across asynchronous processing pipelines.
- **Collaborative Nature**: Participating elements preserve required provenance and audit context according to policy. Telemetry, operational facts, provenance, audit and assurance evidence remain distinct. Being evidence is contextual, not an intrinsic property of every recorded fact; preservation does not establish an assurance conclusion.

29.9.3. Boundary

Telemetry, provenance, audit and assurance evidence remain distinct. Recording evidence neither establishes an assurance conclusion nor automatically requires retention of routine technical activity.

29.9.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.

29.9.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-DF0619B8` (line 10529); `EC-3987B790` (line 10539). Working locators are not identity aliases.

29.9.6. Scope / Contained Elements

Element / page	Type / tier	Element / placement status
Enterprise CT3: Information Attribution & Lineage (p. 871)	enterprise capability CT3	Proposed / Proposed
Enterprise CT3: Governed Activity Audit Evidence (p. 872)	enterprise capability CT3	Proposed / Proposed

29.9.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Person Identity Administration (p. 214)	Proposed
From: enterprise enabled by	BEC CT3: Healthcare Subject Profile Administration (p. 226)	Proposed
From: enterprise enabled by	BEC CT2: Provider Administration (p. 243)	Proposed
From: enterprise enabled by	BEC CT2: Organisation Administration (p. 263)	Proposed
From: enterprise enabled by	BEC CT2: Location Administration (p. 276)	Proposed
From: enterprise enabled by	BEC CT2: Health Service Administration (p. 287)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Device Administration (p. 305)	Proposed
From: enterprise enabled by	BEC CT3: Client Privacy Guardrail Management (p. 237)	Proposed
From: enterprise enabled by	BEC CT2: Health Information Control (p. 715)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Record Administration (p. 443)	Proposed
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
From: enterprise enabled by	BEC CT2: Care Plan Administration (p. 455)	Proposed
From: enterprise enabled by	BEC CT2: Information Design Governance (p. 792)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed
From: enterprise enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed

Relationship	Related element	Assertion status
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Supporting

29.9.8. Enterprise CT3: Information Attribution & Lineage

Property	Value	Meaning
Reference Id	`HCM-EC-information-attribution-lineage`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-07.CT3-EC-INFORMATION-ATTRIBUTION-LINEAGE`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance → Enterprise CT2: Provenance & Traceability	Linked responsibility references; historical records do not assert current ancestry.
Evidence	information-responsibility.md:10; architectural-axioms.md:411	Source standing does not approve the derived assertion.

29.9.8.1. Capability Overview

Lineage explains how an assertion reached its present form without turning every infrastructure event into provenance.

29.9.8.2. Definition — Proposed

Preserve source attribution and significant transformation relationships for managed information.

29.9.8.3. Intent

Explain the information-significant origin and transformation of an assertion.

29.9.8.4. Boundary

Does not infer clinical credibility from a successful transport or establish independent assurance findings.

29.9.8.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:10; architectural-axioms.md:411

29.9.8.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT3: Data Provenance Capture (p. 733)	Proposed
From: enterprise enabled by	BEC CT3: Person Identity Administration (p. 214)	Proposed
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed

29.9.9. Enterprise CT3: Governed Activity Audit Evidence

Property	Value	Meaning
Reference Id	`HCM-EC-governed-activity-audit-evidence`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-07.CT3-EC-GOVERNED-ACTIVITY-AUDIT-EVIDENCE`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance → Enterprise CT2: Provenance & Traceability	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:379	Source standing does not approve the derived assertion.

29.9.9.1. Capability Overview

Denied actions and successful mutations may both require trustworthy evidence; the audited identity and capture authority remain distinct.

29.9.9.2. Definition — Proposed

Capture required significant activity and decision evidence under its applicable governed capture authority.

29.9.9.3. Intent

Preserve required significant action evidence under its proper capture authority.

29.9.9.4. Boundary

Mandatory capture authority is specific to the established audit responsibility, not a platform-wide permission shortcut.

29.9.9.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:379

29.9.9.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Health Information Control (p. 715)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Communication Administration (p. 476)	Proposed

29.10. Enterprise CT2: Operational Assurance

Property	Value	Meaning
Reference Id	`EC-12`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-12`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-12`	Direct to this identity; never reusable.
Evidence	enterprise:202 (p. 1478)	Source standing does not approve the derived assertion.

29.10.1. Capability Overview

Operational Assurance preserves processing integrity under concurrency, duplicate delivery and failure. It brings together resilience, durable ingress acceptance, recovery and operational telemetry so that technical degradation does not silently weaken the managed operation's semantics. Its contribution is collaborative across operational boundaries while remaining a bounded architectural responsibility.

29.10.2. Definition / governed semantic detail

- **Architectural Scope**: Ensure concurrency integrity, system resilience, duplicate detection, exception handling, automated recovery, and operational telemetry across platform operations.
- **Functional Responsibilities**:
  - Enforcing sliding-window duplicate message detection and idempotent suppression.

  - Guaranteeing ingress dual-write safety (durable acceptance before emitting positive application ACK).
  - Managing structured failure recovery, exception escalation, and dead-letter isolation.
  - Emitting platform telemetry, operational health metrics, and queue processing metrics.
- **Collaborative Nature**: Realized collaboratively across boundary gateways, messaging infrastructure, execution workers, and storage layers.

29.10.2.1. EC-12 / EC-14 Boundary

EC-12 retains the established scope and responsibilities above. It may contribute directly to generic operational/processing assurance, limited to that contribution in the approved assurance matrix. It does not own the general semantics of assurance disposition, assurance-specific criteria applicability, evidentiary association, assessment against assurance criteria, adjudication, or independent assurance findings and conclusions. Those distinguishing responsibilities are introduced through EC-14 Service Guardian. Operational health and business-level processing assurance remain distinct; EC-12 is not expanded to absorb EC-14.

29.10.3. Boundary

Operational health is distinct from independent assessment of a governed subject. Assurance-specific criteria applicability, adjudication and conclusions belong to the established EC-14 contribution.

29.10.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.

29.10.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-74BB4AD4` (line 10594); `EC-B0D5A003` (line 10604). Working locators are not identity aliases.

29.10.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Health Information Exchange Enablement (p. 686)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Communication Administration (p. 476)	Proposed
From: enterprise enabled by	BEC CT2: Population Health Information Reporting (p. 817)	Proposed
From: enterprise enabled by	BEC CT2: Work Allocation & Dispatch (p. 607)	Proposed
From: enterprise enabled by	BEC CT2: Patient Transport (p. 632)	Proposed
From: enterprise enabled by	BEC CT2: Clinical Logistics Coordination (p. 639)	Proposed
From: enterprise enabled by	BEC CT2: Discharge Management (p. 645)	Proposed
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Supporting
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Supporting

29.11. Enterprise CT2: Service Guardian

Property	Value	Meaning
Reference Id	`EC-14`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-14`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`EC-14`	Direct to this identity; never reusable.
Evidence	enterprise:224 (p. 1479)	Source standing does not approve the derived assertion.

29.11.1. Capability Overview

Service Guardian supplies the reusable assurance semantics that bind a governed subject to applicable criteria and trustworthy evidence, support assessment and adjudication, and establish reportable findings. Its contribution distinguishes explicit evidence insufficiency from a conclusion about the subject. Existing Enterprise Capabilities contribute collaboratively; EC-14 does not duplicate all their information, policy or execution responsibilities.

Independence concerns control of evaluation and conclusion. A subject may supply evidence but cannot solely determine, suppress or retrospectively change its own assurance outcome.

29.11.2. Definition / governed semantic detail

- **Architectural Scope**: Service Guardian provides the reusable capability to independently establish whether governed service behaviour, state or outcomes remain within defined assurance expectations by associating applicable criteria and trustworthy evidence with a governed subject, assessing that evidence, adjudicating the assessment, and establishing reportable assurance findings and conclusions.
- **Functional Responsibilities**:
  - Supplying assurance-specific semantics for assurance disposition and assurance criteria applicability.
  - Associating a governed subject/context with applicable criteria and sufficient trustworthy evidence for a particular assurance purpose.
  - Assessing evidence against applicable criteria and adjudicating assessed detail to establish reportable findings, exceptions and conclusions.
  - Preserving assurance independence, temporal/version-specific evidence and criteria applicability, and explicit inability to establish a conclusion where evidence is insufficient.
- **Collaborative Nature**: Supplies the assurance-specific semantics required to bind existing reusable capabilities into governed assurance. It contributes directly to Assurance Design, Assurance Criteria Management and Governed Assurance; direct contribution is not sole realisation. The approved matrix preserves the contributions of EC-01 through EC-13.
- **Anti-Responsibilities**: Does not own the underlying governed subject, authoritative governance requirements, or source information used as evidence. Does not perform, manage,

remediate, assign, delegate or operationally escalate the subject activity. Does not replace System Operations or EC-12 Operational Assurance, and does not provide Clinical Services Delivery Assurance.

- **Allocation Boundary**: No strategic logical component, application component, runtime service or implementation construct is allocated EC-14. The catalogue defines a capability, not a software component or deployment boundary.

29.11.2.1. Conceptual Behavioural Sequence

RAD source listing (presentation only; no inferred ancestry).

Define Good Behaviour

↓

Gather Evidence

↓

Assess Evidence against Good Behaviour

↓

Adjudicate Assessed Detail

↓

Report

This sequence explains the capability; it is not an implementation workflow specification or a derived Business Process. “Define Good Behaviour” identifies assurance expectations and criteria; it does not transfer ownership of governing requirements to Service Guardian. Source information ownership remains with its responsible capability; assurance owns its contextual evidentiary association.

The Business Enabling assurance principles govern disposition, generic processing assurance, evidence/criteria temporality, assessment/adjudication, independence, failure/recovery and explicit evidence insufficiency. These semantics are not universally reducible to PASS / FAIL / UNKNOWN and do not imply an assurance information model or persistence mechanism.

29.11.3. Boundary

The capability does not manage or remediate the subject, replace operational assurance, or assure clinical adequacy. No application component, runtime service or deployment boundary is allocated, and the explanatory sequence is not a derived Business Process.

29.11.4. Proposed Placement Rationale

The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.

29.11.5. Prior Candidate Derivation

v0.3 reconciliation: `EC-74BB4AD4` (line 10594); `EC-B0D5A003` (line 10604). Working locators are not identity aliases.

29.11.6. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise contribution	BEC CT3: Assurance Design (p. 824)	Established / Direct
From: enterprise contribution	BEC CT3: Assurance Criteria Administration (p. 828)	Established / Direct

29.12. Enterprise CT2: Information Authority Qualification

Property	Value	Meaning
Reference Id	`HCM-EC-information-authority-qualification`	Stable concept identity.
Reconciliation standing	current	Current reviewed hierarchy.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	enterprise capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Structural Canonical ID	`CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-INFORMATION-AUTHORITY-QUALIFICATION`	Current proposed context where marked Proposed.
Current context	Enterprise CT1: Trust, Accountability & Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:303; information-responsibility.md:10; calliope-models-and-datasets.md:120	Source standing does not approve the derived assertion.

29.12.1. Capability Overview

Identity correlation, clinical-history synthesis, reference-content use and assurance evidence all need to distinguish the standing of an assertion from technical acceptance. This recurring ability is insufficiently explicit in the existing fourteen EC definitions and is proposed as a new peer responsibility.

29.12.2. Definition — Proposed

Represent and qualify the source-authority and credibility standing of information independently of transport, persistence and concurrency.

29.12.3. Intent

Keep source credibility explicit even when the technical handling of competing assertions is equally valid.

29.12.4. Boundary

This is not independent service assurance, a universal conflict arbiter or clinical truth determination. Existing EC-13 governs authority nomenclature; this capability applies and preserves the qualified standing.

29.12.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

29.12.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:303; information-responsibility.md:10; calliope-models-and-datasets.md:120

29.12.7. Relationships / Architectural Relevance

Relationship	Related element	Assertion status
From: enterprise enabled by	BEC CT2: Patient Clinical Record (p. 656)	Proposed
From: enterprise enabled by	BEC CT3: Person Identity Administration (p. 214)	Proposed
From: enterprise enabled by	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed

Part V.

Architecture registers and provenance

30. Human-reviewed BEC reconciliation

The explicit human-reviewed candidate governs this bounded reconciliation. It does not accept this publication into RADS. AX-14 preserves semantic distinctions; AX-17 retains unadjudicated responsibility placement; AX-18 prevents information administration from conferring authority over clinical or enterprise activity. BC and EC architecture have not been independently redesigned.

Primary disposition	Predecessor count
renamed	27
retained unchanged	7
consolidated	14
retained but outside R1.x/R2.x	9
relocated	46
decomposed	2
superseded by reviewed capability	21
genuine unresolved conflict requiring human adjudication	11

30.1. Release scope annotations

The following annotation is independent of architectural status. Inherited scope applies to the entire stated branch. Existing compatible decomposition is retained only where useful; an empty branch does not require invented CT3 abilities.

Capability	Release scope
BEC CT2: Client-Centred Administration (p. 328)	Not Addressed in R1.x / R2.x
BEC CT2: Episode & Encounter Administration (p. 363)	Not Addressed in R1.x / R2.x
BEC CT2: Diagnostic Administration (p. 399)	Not Addressed in R1.x / R2.x
BEC CT2: Medication Administration (p. 417)	Not Addressed in R1.x / R2.x
BEC CT2: Procedure Administration (p. 435)	Not Addressed in R1.x / R2.x
BEC CT2: Clinical Communication Administration (p. 476)	Not Addressed in R1.x / R2.x
BEC CT2: Funding & Coverage Administration (p. 487)	Not Addressed in R1.x / R2.x
BEC CT2: Clinical Charging & Claims Administration (p. 489)	Not Addressed in R1.x / R2.x
BEC CT2: Clinical Training & Education (p. 618)	Not Addressed in R1.x / R2.x
BEC CT1: Clinical Service Dependency Management (p. 491)	Not Addressed in R1.x / R2.x
BEC CT2: Service Establishment Support (p. 586)	Not Addressed in R1.x / R2.x

30.2. Relocated service-context responsibilities

These surviving responsibilities concern care information or coordination rather than operational prerequisites. They have not been converted into dependency-management abilities.

Predecessor	Surviving responsibility	Reviewed parent
Primary Care	BEC CT3: Primary Care Coordination (p. 669)	Patient Clinical Record
Acute Care	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Episode & Encounter Administration
Emergency Care	BEC CT3: Emergency Care Progression (p. 554)	Emergency Department Operations
Inpatient Care	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Ward Operations

Predecessor	Surviving responsibility	Reviewed parent
Diagnostic Services	BEC CT3: Diagnostic Service Integration (p. 401)	Diagnostic Administration
Medication Therapy	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Medication Administration
Preventive Care	BEC CT3: Screening and Immunisation Enablement (p. 329)	Client-Centred Administration
Community Care	BEC CT3: Community Care Coordination (p. 460)	Care Plan Administration
Outreach Care	BEC CT3: Outreach Care Continuity (p. 466)	Care Plan Administration
Remote & Virtual Care	BEC CT3: Remote Patient Monitoring (p. 675)	Patient Clinical Record

30.3. Consent and unresolved conflicts

Preserved; restriction/filter/disclosure references the existing client-associated responsibility. No duplicate consent capability introduced. BEC CT3: Client Privacy Guardrail Management.

HUMAN ADJUDICATION REQUIRED: reviewed name Health Information Exchange duplicates the Canonical BC name under CM-R05. The existing BEC display qualification is retained; reviewed_name records the exact task wording. Identities and BC/BEC semantics remain distinct.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Credential Management (p. 899). Canonical operational badges/physical-access credentials are distinct from reviewed clinical-service credential definitions.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Health Information Communication (p. 890). Canonical boundary protocol mediation and technical transmission are distinct from reviewed patient-facing communication.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Workflow & Activity Coordination (p. 891). Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Governed Assurance (p. 898). Canonical assurance against a governed subject is not independent review of the assurance service itself.

HUMAN ADJUDICATION REQUIRED: BEC CT2: Independent Assurance Evaluation Enablement (p. 958). The previous grouping frames Canonical subject-assurance execution; it cannot be equated with review of the assurance service itself.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Information Authority Representation (p. 959). Source credibility and authority standing require an explicit home; information-asset tags alone do not establish that responsibility.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Significant Activity Audit Capture (p. 961). The reviewed hierarchy does not allocate general mandatory authoritative audit capture; disclosure or provenance alone is not an equivalent responsibility.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Governance Requirement Publication Support (p. 962). Requirement publication/authority is not assurance-service review or information-model design; the reviewed hierarchy does not allocate a home.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Entity-Specific Operational Coordination (p. 963). Internal entity-relative progression is not the reviewed external work bridge; a current BEC destination has not been adjudicated.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Governed Operational Activity Design (p. 964). Internal execution-envelope design is distinct from governance of work across an external execution boundary.

HUMAN ADJUDICATION REQUIRED: BEC CT3: Service Offering Readiness Coordination (p. 966). Dependency evidence material retained at the out-of-scope CT1. A proposed determination of collective go/no-go readiness is not inherited as authority by dependency tracking.

30.4. Complete predecessor ledger

Identity-level decisions, predecessor identities, semantic distributions, every changed structural ID and direct alias, and Feature/behaviour ancestry changes are retained in bec-reconciliation-ledger.json and the task report. The historical entries preserve their exact pre-review records.

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-healthcare-entity-information-enablement` Healthcare Entity Information Enablement	renamed	BEC CT1: Entity Information Enablement (p. 211)
`HCM-BEC-client-administration` Client Administration	retained unchanged	BEC CT2: Client Administration (p. 212)
`HCM-BEC-person-identity` Person Identity	renamed	BEC CT3: Person Identity Administration (p. 214)
`HCM-BEC-healthcare-subject` Healthcare Subject	renamed	BEC CT3: Healthcare Subject Profile Administration (p. 226)
`HCM-BEC-client-relationship` Client Relationship	renamed	BEC CT3: Client Relationship Administration (p. 232)
`HCM-BEC-client-privacy` Client Privacy	renamed	BEC CT3: Client Privacy Guardrail Management (p. 237)
`HCM-BEC-provider-administration` Provider Administration	retained unchanged	BEC CT2: Provider Administration (p. 243)
`HCM-BEC-practitioner` Practitioner	renamed	BEC CT3: Practitioner Profile Administration (p. 245)
`HCM-BEC-practitioner-role` Practitioner Role	consolidated	BEC CT3: Practitioner Role Administration (p. 249)
`HCM-BEC-organisation-administration` Organisation Administration	retained unchanged	BEC CT2: Organisation Administration (p. 263)
`HCM-BEC-healthcare-organisation` Healthcare Organisation	consolidated	BEC CT3: Healthcare Organisation Profile Administration (p. 265)
`HCM-BEC-location-administration` Location Administration	retained unchanged	BEC CT2: Location Administration (p. 276)
`HCM-BEC-healthcare-location` Healthcare Location	consolidated	BEC CT3: Healthcare Location Profile Administration (p. 277)
`HCM-BEC-care-place-definition` Care-Place Definition	renamed	BEC CT3: Care-Place Definition Administration (p. 281)
`HCM-BEC-health-service-administration` Health Service Administration	retained unchanged	BEC CT2: Health Service Administration (p. 287)
`HCM-BEC-healthcare-service-definition` Healthcare Service Definition	consolidated	BEC CT3: Healthcare Service Profile Administration (p. 289)
`HCM-BEC-health-product-administration` Health Product Administration	renamed	BEC CT2: Health Materiel Administration (p. 300)
`HCM-BEC-health-product-definition` Health Product Definition	renamed	BEC CT3: Health Materiel Profile Administration (p. 301)
`HCM-BEC-product-formulary-catalogue` Product Formulary & Catalogue	renamed	BEC CT3: Health Materiel Catalogue & Formulary Administration (p. 302)
`HCM-BEC-product-batch-serial-tracking` Product Batch & Serial Tracking	renamed	BEC CT3: Health Materiel Traceability Administration (p. 304)
`HCM-BEC-clinical-device-administration` Clinical Device Administration	retained unchanged	BEC CT2: Clinical Device Administration (p. 305)
`HCM-BEC-device-definition` Device Definition	consolidated	BEC CT3: Clinical Device Profile Administration (p. 307)

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-care-administration-information-enablement` Care Administration Information Enablement	renamed	BEC CT1: Care Administration Enablement (p. 326)
`HCM-BEC-client-centred-administration` Client-Centred Administration	retained but outside R1.x/R2.x	BEC CT2: Client-Centred Administration (p. 328)
`HCM-BEC-referral-administration` Referral Administration	relocated	BEC CT2: Referral Administration (p. 342)
`HCM-BEC-scheduling-administration` Scheduling Administration	relocated	BEC CT2: Scheduling Administration (p. 355)
`HCM-BEC-episode-encounter-administration` Episode & Encounter Administration	retained but outside R1.x/R2.x	BEC CT2: Episode & Encounter Administration (p. 363)
`HCM-BEC-order-administration` Order Administration	relocated	BEC CT2: Order Administration (p. 383)
`HCM-BEC-diagnostic-administration` Diagnostic Administration	retained but outside R1.x/R2.x	BEC CT2: Diagnostic Administration (p. 399)
`HCM-BEC-medication-administration` Medication Administration	retained but outside R1.x/R2.x	BEC CT2: Medication Administration (p. 417)
`HCM-BEC-procedure-administration` Procedure Administration	retained but outside R1.x/R2.x	BEC CT2: Procedure Administration (p. 435)
`HCM-BEC-clinical-record-administration` Clinical Record Administration	relocated	BEC CT2: Clinical Record Administration (p. 443)
`HCM-BEC-care-plan-administration` Care Plan Administration	relocated	BEC CT2: Care Plan Administration (p. 455)
`HCM-BEC-clinical-communication-administration` Clinical Communication Administration	retained but outside R1.x/R2.x	BEC CT2: Clinical Communication Administration (p. 476)
`HCM-BEC-funding-coverage-administration` Funding & Coverage Administration	retained but outside R1.x/R2.x	BEC CT2: Funding & Coverage Administration (p. 487)
`HCM-BEC-clinical-charging-claims-administration` Clinical Charging & Claims Administration	retained but outside R1.x/R2.x	BEC CT2: Clinical Charging & Claims Administration (p. 489)
`HCM-BEC-clinical-assessment` Clinical Assessment	relocated	BEC CT2: Clinical Assessment (p. 492)
`HCM-BEC-ambulatory-care` Ambulatory Care	relocated	BEC CT2: Ambulatory Care (p. 498)
`HCM-BEC-surgical-procedural-care` Surgical & Procedural Care	relocated	BEC CT2: Surgical & Procedural Care (p. 499)
`HCM-BEC-therapeutic-services` Therapeutic Services	relocated	BEC CT2: Therapeutic Services (p. 501)
`HCM-BEC-allied-health` Allied Health	relocated	BEC CT2: Allied Health (p. 504)
`HCM-BEC-rehabilitation` Rehabilitation	relocated	BEC CT2: Rehabilitation (p. 506)
`HCM-BEC-care-operations-coordination-enablement` Care Operations Coordination Enablement	renamed	BEC CT1: Health Service Operations Enablement (p. 512)
`HCM-BEC-clinic-practice-operations` Clinic & Practice Operations	relocated	BEC CT2: Clinic & Practice Operations (p. 514)
`HCM-BEC-ward-operations` Ward Operations	relocated	BEC CT2: Ward Operations (p. 524)

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-theatre-operations` Theatre Operations	relocated	BEC CT2: Theatre Operations (p. 538)
`HCM-BEC-emergency-department-operations` Emergency Department Operations	relocated	BEC CT2: Emergency Department Operations (p. 548)
`HCM-BEC-outpatient-operations` Outpatient Operations	relocated	BEC CT2: Outpatient Operations (p. 561)
`HCM-BEC-bed-care-place-management` Bed & Care-Place Management	relocated	BEC CT2: Bed & Care-Place Management (p. 569)
`HCM-BEC-service-capacity-management` Service Capacity Management	relocated	BEC CT2: Service Capacity Management (p. 581)
`HCM-BEC-clinical-workforce-management` Clinical Workforce Management	renamed	BEC CT2: Clinical Workforce Capability Monitoring (p. 587)
`HCM-BEC-clinical-rostering` Clinical Rostering	relocated	BEC CT2: Clinical Rostering (p. 589)
`HCM-BEC-mobile-staff-management` Mobile Staff Management	relocated	BEC CT2: Mobile Staff Management (p. 591)
`HCM-BEC-on-call-management` On-Call Management	relocated	BEC CT2: On-Call Management (p. 601)
`HCM-BEC-work-allocation-dispatch` Work Allocation & Dispatch	relocated	BEC CT2: Work Allocation & Dispatch (p. 607)
`HCM-BEC-clinical-training-education` Clinical Training & Education	retained but outside R1.x/R2.x	BEC CT2: Clinical Training & Education (p. 618)
`HCM-BEC-clinical-competency-management` Clinical Competency Management	renamed	BEC CT2: Clinical Competency Administration (p. 619)
`HCM-BEC-clinical-qualification-management` Clinical Qualification Management	renamed	BEC CT2: Clinical Qualification Administration (p. 621)
`HCM-BEC-clinical-supervision` Clinical Supervision	relocated	BEC CT2: Clinical Supervision (p. 630)
`HCM-BEC-patient-transport` Patient Transport	relocated	BEC CT2: Patient Transport (p. 632)
`HCM-BEC-clinical-logistics-coordination` Clinical Logistics Coordination	relocated	BEC CT2: Clinical Logistics Coordination (p. 639)
`HCM-BEC-discharge-management` Discharge Management	relocated	BEC CT2: Discharge Management (p. 645)
`HCM-BEC-governed-health-information-enablement` Governed Health Information Enablement	renamed	BEC CT1: Shared Information & Coordination Enablement (p. 654)
`HCM-BEC-patient-clinical-record` Patient Clinical Record	relocated	BEC CT2: Patient Clinical Record (p. 656)
`HCM-BEC-associated-clinical-binary-content-enablement` Associated Clinical Binary Content Enablement	renamed	BEC CT3: Linked Information Curation (p. 663)
`HCM-BEC-health-information-exchange` Health Information Exchange Enablement	renamed	BEC CT2: Health Information Exchange Enablement (p. 686)
`HCM-BEC-health-information-access` Health Information Access	relocated	BEC CT2: Health Information Access (p. 701)
`HCM-BEC-health-information-control` Health Information Control	relocated	BEC CT2: Health Information Control (p. 715)
`HCM-BEC-information-lineage-stewardship` Information Lineage Stewardship	renamed	BEC CT3: Data Provenance Capture (p. 733)

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-clinical-knowledge-services` Clinical Knowledge Services	relocated	BEC CT2: Clinical Knowledge Services (p. 742)
`HCM-BEC-governed-reference-dataset-administration` Governed Reference Dataset Administration	relocated	BEC CT3: Governed Reference Dataset Administration (p. 748)
`HCM-BEC-clinical-collaboration` Clinical Collaboration	relocated	BEC CT2: Clinical Collaboration (p. 756)
`HCM-BEC-general-collaboration` General Collaboration	relocated	BEC CT2: General Collaboration (p. 768)
`HCM-BEC-calendar-management` Calendar Management	relocated	BEC CT2: Calendar Management (p. 779)
`HCM-BEC-presentation-services` Presentation Services	relocated	BEC CT2: Presentation Services (p. 786)
`HCM-BEC-information-design-governance` Information Design Governance	relocated	BEC CT2: Information Design Governance (p. 792)
`HCM-BEC-secondary-health-information-use-enablement` Secondary Health Information Use Enablement	renamed	BEC CT1: Health Informatics Management (p. 811)
`HCM-BEC-authorised-research-information-provision` Authorised Research Information Provision	renamed	BEC CT2: Research Information Curation (p. 812)
`HCM-BEC-population-health-notification-enablement` Population Health Notification Enablement	renamed	BEC CT2: Population Health Information Reporting (p. 817)
`HCM-BEC-service-planning-information-provision` Service Planning Information Provision	renamed	BEC CT2: Service Planning Data Management (p. 818)
`HCM-BEC-independent-service-assurance-enablement` Independent Service Assurance Enablement	renamed	BEC CT1: Independent Assurance Enablement (p. 821)
`HCM-BEC-assurance-specification-enablement` Assurance Specification Enablement	renamed	BEC CT2: Assurance Specification (p. 822)
`HCM-BEC-assurance-design` Assurance Design	retained unchanged	BEC CT3: Assurance Design (p. 824)
`HCM-BEC-assurance-criteria-management` Assurance Criteria Management	renamed	BEC CT3: Assurance Criteria Administration (p. 828)
`HCM-BEC-provider-relationship` Provider Relationship	consolidated	BEC CT3: Provider Relationship (p. 925)
`HCM-BEC-provider-engagement` Provider Engagement	decomposed	BEC CT3: Provider Engagement (p. 926)
`HCM-BEC-organisational-structure` Organisational Structure	consolidated	BEC CT3: Organisational Structure (p. 927)
`HCM-BEC-organisation-contact` Organisation Contact	consolidated	BEC CT3: Organisation Contact (p. 928)
`HCM-BEC-location-structure` Location Structure	consolidated	BEC CT3: Location Structure (p. 930)
`HCM-BEC-service-provision-relationship` Service Provision Relationship	consolidated	BEC CT3: Service Provision Relationship (p. 931)

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-service-availability-definition` Service Availability Definition	consolidated	BEC CT3: Service Availability Definition (p. 932)
`HCM-BEC-service-eligibility-definition` Service Eligibility Definition	consolidated	BEC CT3: Service Eligibility Definition (p. 933)
`HCM-BEC-device-instance` Device Instance	consolidated	BEC CT3: Device Instance (p. 934)
`HCM-BEC-device-association` Device Association	decomposed	BEC CT3: Device Association (p. 935)
`HCM-BEC-device-connectivity` Device Connectivity	consolidated	BEC CT3: Device Connectivity (p. 936)
`HCM-BEC-clinical-resource-management` Clinical Resource Management	superseded by reviewed capability	BEC CT3: Clinical Resource Management (p. 889)
`HCM-BEC-primary-care` Primary Care	relocated	BEC CT3: Primary Care Coordination (p. 669)
`HCM-BEC-acute-care` Acute Care	relocated	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)
`HCM-BEC-emergency-care` Emergency Care	relocated	BEC CT3: Emergency Care Progression (p. 554)
`HCM-BEC-inpatient-care` Inpatient Care	relocated	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)
`HCM-BEC-diagnostic-services` Diagnostic Services	relocated	BEC CT3: Diagnostic Service Integration (p. 401)
`HCM-BEC-medication-therapy` Medication Therapy	relocated	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)
`HCM-BEC-preventive-care` Preventive Care	relocated	BEC CT3: Screening and Immunisation Enablement (p. 329)
`HCM-BEC-community-care` Community Care	relocated	BEC CT3: Community Care Coordination (p. 460)
`HCM-BEC-outreach-care` Outreach Care	relocated	BEC CT3: Outreach Care Continuity (p. 466)
`HCM-BEC-remote-virtual-care` Remote & Virtual Care	relocated	BEC CT3: Remote Patient Monitoring (p. 675)
`HCM-BEC-credential-management` Credential Management	genuine unresolved conflict requiring human adjudication	BEC CT3: Credential Management (p. 899)
`HCM-BEC-health-information-communication` Health Information Communication	genuine unresolved conflict requiring human adjudication	BEC CT3: Health Information Communication (p. 890)
`HCM-BEC-workflow-activity-coordination` Workflow & Activity Coordination	genuine unresolved conflict requiring human adjudication	BEC CT3: Workflow & Activity Coordination (p. 891)
`HCM-BEC-governed-assurance` Governed Assurance	genuine unresolved conflict requiring human adjudication	BEC CT3: Governed Assurance (p. 898)
`HCM-BEC-independent-assurance-evaluation-enablement` Independent Assurance Evaluation Enablement	genuine unresolved conflict requiring human adjudication	BEC CT2: Independent Assurance Evaluation Enablement (p. 958)
`HCM-BEC-information-authority-representation` Information Authority Representation	genuine unresolved conflict requiring human adjudication	BEC CT3: Information Authority Representation (p. 959)
`HCM-BEC-significant-activity-audit-capture` Significant Activity Audit Capture	genuine unresolved conflict requiring human adjudication	BEC CT3: Significant Activity Audit Capture (p. 961)

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-governance-requirement-publication-support` Governance Requirement Publication Support	genuine unresolved conflict requiring human adjudication	BEC CT3: Governance Requirement Publication Support (p. 962)
`HCM-BEC-entity-specific-operational-coordination` Entity-Specific Operational Coordination	genuine unresolved conflict requiring human adjudication	BEC CT3: Entity-Specific Operational Coordination (p. 963)
`HCM-BEC-governed-operational-activity-design` Governed Operational Activity Design	genuine unresolved conflict requiring human adjudication	BEC CT3: Governed Operational Activity Design (p. 964)
`HCM-BEC-service-offering-readiness-coordination` Service Offering Readiness Coordination	genuine unresolved conflict requiring human adjudication	BEC CT3: Service Offering Readiness Coordination (p. 966)
`HCM-BEC-care-context-integration-enablement` Care Context Integration Enablement	superseded by reviewed capability	BEC CT1: Care Context Integration Enablement (p. 923)
`HCM-BEC-human-collaboration-work-enablement` Human Collaboration & Work Enablement	superseded by reviewed capability	BEC CT1: Human Collaboration & Work Enablement (p. 924)
`HCM-BEC-care-access-transaction-enablement` Care Access Transaction Enablement	superseded by reviewed capability	BEC CT2: Care Access Transaction Enablement (p. 937)
`HCM-BEC-care-fulfilment-administration-enablement` Care Fulfilment Administration Enablement	superseded by reviewed capability	BEC CT2: Care Fulfilment Administration Enablement (p. 939)
`HCM-BEC-care-documentation-communication-enablement` Care Documentation & Communication Enablement	superseded by reviewed capability	BEC CT2: Care Documentation & Communication Enablement (p. 940)
`HCM-BEC-care-financial-information-enablement` Care Financial Information Enablement	superseded by reviewed capability	BEC CT2: Care Financial Information Enablement (p. 941)
`HCM-BEC-longitudinal-community-care-integration` Longitudinal & Community Care Integration	superseded by reviewed capability	BEC CT2: Longitudinal & Community Care Integration (p. 942)
`HCM-BEC-facility-episodic-care-integration` Facility & Episodic Care Integration	superseded by reviewed capability	BEC CT2: Facility & Episodic Care Integration (p. 943)
`HCM-BEC-specialist-clinical-context-integration` Specialist Clinical Context Integration	superseded by reviewed capability	BEC CT2: Specialist Clinical Context Integration (p. 945)
`HCM-BEC-care-setting-operations-enablement` Care Setting Operations Enablement	superseded by reviewed capability	BEC CT2: Care Setting Operations Enablement (p. 946)
`HCM-BEC-care-resource-capacity-coordination` Care Resource & Capacity Coordination	superseded by reviewed capability	BEC CT2: Care Resource & Capacity Coordination (p. 947)
`HCM-BEC-operational-workforce-coordination` Operational Workforce Coordination	superseded by reviewed capability	BEC CT2: Operational Workforce Coordination (p. 948)
`HCM-BEC-workforce-qualification-development-enablement` Workforce Qualification & Development Enablement	superseded by reviewed capability	BEC CT2: Workforce Qualification & Development Enablement (p. 949)

Predecessor identity	Disposition	Current / retained entry
`HCM-BEC-care-movement-transition-coordination` Care Movement & Transition Coordination	superseded by reviewed capability	BEC CT2: Care Movement & Transition Coordination (p. 950)
`HCM-BEC-clinical-information-stewardship-enablement` Clinical Information Stewardship Enablement	superseded by reviewed capability	BEC CT2: Clinical Information Stewardship Enablement (p. 952)
`HCM-BEC-health-information-connectivity-enablement` Health Information Connectivity Enablement	superseded by reviewed capability	BEC CT2: Health Information Connectivity Enablement (p. 953)
`HCM-BEC-semantic-reference-knowledge-enablement` Semantic & Reference Knowledge Enablement	superseded by reviewed capability	BEC CT2: Semantic & Reference Knowledge Enablement (p. 954)
`HCM-BEC-information-protection-accountability-enablement` Information Protection & Accountability Enablement	superseded by reviewed capability	BEC CT2: Information Protection & Accountability Enablement (p. 955)
`HCM-BEC-collaborative-interaction-enablement` Collaborative Interaction Enablement	superseded by reviewed capability	BEC CT2: Collaborative Interaction Enablement (p. 956)
`HCM-BEC-operational-activity-temporal-enablement` Operational Activity & Temporal Enablement	superseded by reviewed capability	BEC CT2: Operational Activity & Temporal Enablement (p. 957)

31. Preserved responsibilities and reconciliation history

These entries are deliberately outside the current BEC hierarchy. Superseded structural records retain their identities, labels, last allocated structural IDs and exact prior semantics. Pending responsibilities retain their Canonical or Proposed semantic standing and owned material while a genuine placement or meaning conflict awaits human adjudication. Their historical placements are not current decomposition assertions. This register is evidence and an explicit unresolved register, not an additional BEC branch.

31.1. BEC CT3: Clinical Resource Management

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-resource-management`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Care Resource & Capacity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Harmonia-Relevant	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:454 (p. 1375); enabling:512 (p. 1377); hso:169 (p. 1460)	Source standing does not approve the derived assertion.

31.1.1. Capability Overview

Clinical Resource Management integrates location and allocation information for shared clinical equipment. It makes the current operational resource context available rather than treating device inventory as proof that equipment can be used for a particular need. The responsibility supports coordination while retaining the distinction between resource state and a service's capacity.

31.1.2. Boundary

Equipment tracking does not acquire enterprise asset ownership, maintenance responsibility or clinical authority to use it.

31.1.3. Harmonia Architectural Relevance

Dynamic equipment context materially supports operational coordination across care environments.

31.1.4. Proposed Placement Rationale

The responsibility concerns relate operational care-place and equipment state to the coordinated view of service demand and capacity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

31.1.5. Reviewed successors

BEC CT3: Device Utilisation Administration (p. 311); BEC CT1: Clinical Service Dependency Management (p. 491)

31.2. BEC CT3: Health Information Communication

Property	Value	Meaning
Reference Id	`HCM-BEC-health-information-communication`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Connectivity Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:568 (p. 1379); enabling:626 (p. 1380); ise:92 (p. 1467)	Source standing does not approve the derived assertion.

31.2.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical boundary protocol mediation and technical transmission are distinct from reviewed patient-facing communication.

31.2.2. Capability Overview

Health Information Communication supplies standards-based transmission mediation between participating parties. It addresses protocol-facing transaction coordination and technical error

handling while the business interaction retains its own meaning and authority. This differentiates the communication mechanism from the exchange model and from the clinical or operational purpose of the message.

31.2.3. Boundary

Transport mediation does not own every communicated interaction or payload. External contracts must preserve standards semantics and exclude private operational metadata.

31.2.4. Harmonia Architectural Relevance

Standards-conformant communication boundaries are a direct interoperability responsibility.

31.2.5. Proposed Placement Rationale

The responsibility concerns make governed health information discoverable and exchangeable under explicit internal and external contracts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

31.3. BEC CT3: Workflow & Activity Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-workflow-activity-coordination`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Harmonia-Core	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:573 (p. 1379); enabling:651 (p. 1381); ise:171 (p. 1469)	Source standing does not approve the derived assertion.

31.3.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.3.2. Capability Overview

Workflow & Activity Coordination provides governed progression for three distinct work meanings: human operational Work Orders, human review or decision To Dos, and non-human Synthetic Tasks. It supports dispatch, state and deadline oversight without collapsing those meanings into a generic technical task. The common coordination responsibility makes progression visible while preserving who has authority to perform or decide the underlying activity.

31.3.3. Intent

Keep assigned activity, acknowledgement, progress and outcome distinguishable across human and automated work.

31.3.4. Boundary — v0.1 explanation

Coordination does not confer clinical work ownership. Synthetic Tasks are not external clinical worklists, and the incomplete stalled/failed Process paths remain unresolved.

31.3.5. Harmonia Architectural Relevance

Governed activity progression is a substantive Core responsibility; information traceability preserves archetype, work and undertaking distinctions without transferring outcome authority.

31.3.6. Proposed Placement Rationale

The responsibility concerns progress governed operational work in identifiable activity and temporal contexts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

31.4. Feature: Standards-Based Boundary Exchange

Property	Value	Meaning
Reference Id	`FEAT-ISE-10`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION.FEAT-ISE-10`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Connectivity Enablement → BEC CT3: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	enabling:628 (p. 1380)	Source standing does not approve the derived assertion.

31.4.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical boundary protocol mediation and technical transmission are distinct from reviewed patient-facing communication.

31.4.2. Feature Overview

Boundary exchange preserves the standards contract understood by the external participant. Harmonia's internal management metadata must not leak into that contract, and publication must construct an allowed external representation without destructively changing the internally managed information.

31.4.3. Definition / governed semantic detail

Provide standards-conformant boundary exchange for healthcare messaging and resource representations.

31.5. Feature: Security Context Propagation

Property	Value	Meaning
Reference Id	`FEAT-ISE-12`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-ISE-12`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:633 (p. 1381)	Source standing does not approve the derived assertion.

31.5.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: general platform security/evidence Feature has no adjudicated patient-specific CT3 frame; established Health Information Control responsibility is preserved.

31.5.2. Feature Overview

Security-context propagation carries attributable operational authority across asynchronous boundaries. Its responsibility is preservation of established context, distinct from evaluating a consent constraint; shared security subject matter is insufficient to assign that Function to this Feature.

31.5.3. Definition / governed semantic detail

Propagate tamper-evident, attributable security context across asynchronous processing boundaries.

31.6. Feature: Non-PHI Compliance Auditing

Property	Value	Meaning
Reference Id	`FEAT-ISE-13`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-ISE-13`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:634 (p. 1381)	Source standing does not approve the derived assertion.

31.6.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: general platform security/evidence Feature has no adjudicated patient-specific CT3 frame; established Health Information Control responsibility is preserved.

31.6.2. Feature Overview

Security-significant audit evidence makes governed actions attributable and verifiable while respecting PHI restrictions. The record concerns meaningful security facts under policy rather than routine implementation events, and recording it does not independently establish an assurance conclusion.

31.6.3. Definition / governed semantic detail

Generate tamper-evident, verifiable audit records of security-significant events without emitting unmasked PHI.

31.7. Feature: Work Order Progression

Property	Value	Meaning
Reference Id	`FEAT-ISE-20`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-20`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:653 (p. 1381)	Source standing does not approve the derived assertion.

31.7.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.7.2. Feature Overview

Work Order progression coordinates human operational doing as a distinct work meaning. Dispatch, acceptance and completion describe the undertaking without making the workflow engine the authority over clinical work or treating a receipt as performance.

31.7.3. Definition / governed semantic detail

Manage execution states and dispatch of human operational work orders.

31.8. Feature: To Do Decision & Approval Coordination

Property	Value	Meaning
Reference Id	`FEAT-ISE-21`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.

Property	Value	Meaning
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-21`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:654 (p. 1381)	Source standing does not approve the derived assertion.

31.8.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.8.2. Feature Overview

To Do coordination supports human review, sign-off and decision activity. Its progression can expose whether the review item has been handled while leaving the meaning and authority of the professional or governance decision with the responsible participant.

31.8.3. Definition / governed semantic detail

Coordinate human clinical review items, sign-offs, and approval workflows.

31.9. Feature: Synthetic Task Orchestration

Property	Value	Meaning
Reference Id	`FEAT-ISE-22`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-22`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:655 (p. 1381)	Source standing does not approve the derived assertion.

31.9.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.9.2. Feature Overview

Synthetic Task orchestration governs non-human executable work and its sequencing. This technical work meaning remains distinct from human Work Orders, clinical worklists and external systems' generic task terminology; incomplete stalled/failed Process paths are not supplied by convention.

31.9.3. Definition / governed semantic detail

Execute non-human, automated platform activity units and sequence orchestration.

31.10. Feature: Workflow Timeout & Escalation Oversight

Property	Value	Meaning
Reference Id	`FEAT-ISE-23`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-23`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	enabling:656 (p. 1381)	Source standing does not approve the derived assertion.

31.10.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.10.2. Feature Overview

Timeout oversight observes deadlines and initiates the established coordination response to stalled activity. It is management of progression rather than an assurance conclusion, and escalation does not grant the coordinator authority over the clinical meaning of the underlying work.

31.10.3. Definition / governed semantic detail

Supervise operational deadlines, executing automated timeout notifications and supervisory escalations.

31.11. BEC CT3: Governed Assurance

Property	Value	Meaning
Reference Id	`HCM-BEC-governed-assurance`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	enabling:694 (p. 1383)	Source standing does not approve the derived assertion.

31.11.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.11.2. Capability Overview

Governed Assurance independently evaluates a subject through applicable criteria and trustworthy evidence, establishing explicit findings and conclusions. Context establishment associates source-owned information with a particular assurance purpose; assessment evaluates it; adjudication establishes the supported outcome. Insufficient evidence remains a meaningful limit rather than a forced positive or negative result.

The capability can manage its own assurance activity without managing the subject. Request, progression status and established outcome are different client concerns; the requester does not thereby control the evaluation or gain unrestricted status access.

31.11.3. Definition / governed semantic detail

The capability to independently evaluate a governed subject against applicable assurance criteria through the assembly and evaluation of sufficient trustworthy evidence, establishing explicit findings, exceptions and assurance conclusions.

31.11.4. Boundary

No remediation, assignment or operational escalation of the subject is acquired. Financial governance is deliberately excluded; assurance does not recursively assure its own execution, and no component allocation is established.

31.11.5. Harmonia Architectural Relevance — v0.1 explanation

Formal classification remains Unresolved. The approved independent-assurance responsibility and contribution model establish material purpose without resolving that metadata dimension.

31.11.6. Proposed Placement Rationale

The responsibility concerns associate trustworthy evidence, assess it and adjudicate independently reportable findings about governed service behaviour. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

31.12. BEC CT3: Credential Management

Property	Value	Meaning
Reference Id	`HCM-BEC-credential-management`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CREDENTIAL-MANAGEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Workforce Qualification & Development Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Unresolved in the semantic baseline	Independent of element/placement status; never inherited from an ancestor.
Evidence	hso:239 (p. 1462)	Source standing does not approve the derived assertion.

31.12.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical operational badges/physical-access credentials are distinct from reviewed clinical-service credential definitions.

31.12.2. Capability Overview

Credential Management concerns operational badges and physical-access credentials used in authorised operational participation. It retains a distinct Domain03 responsibility even though Strategy has no matching catalogue entry. This prevents the word credential from merging physical-access context with the professional qualifications and competency evidence checked by Clinical Qualification Management.

31.12.3. Boundary

No Strategy correspondence or Feature association is inferred from name similarity. Operational credentials do not themselves establish professional clinical qualification or privilege.

31.12.4. Harmonia Architectural Relevance — v0.1 explanation

Relevance remains Unresolved. Established operational behaviour provides context for possible enablement but does not authorise a formal classification.

31.12.5. Proposed Placement Rationale

The responsibility concerns represent learning, competence, professional-standing checks and separately governed operational access qualifications. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

31.12.6. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.13. Function: Verify Operational Access Credential

Property	Value	Meaning
Reference Id	`HCM-FN-verify-operational-access-credential`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CREDENTIAL-MANAGEMENT.FN-VERIFY-OPERATIONAL-ACCESS-CREDENTIAL`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Workforce Qualification & Development Enablement → BEC CT3: Credential Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	hso:243 (p. 1462); hso:241 (p. 1462)	Source standing does not approve the derived assertion.

31.13.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical operational badges/physical-access credentials are distinct from reviewed clinical-service credential definitions.

31.13.2. Definition / governed semantic detail

Verifies physical access badges, specialty ward entry authorizations, and restricted area permissions.

31.14. Service: Operational Credential Checkpoint Service

Property	Value	Meaning
Reference Id	`HCM-SV-operational-credential-checkpoint-service`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CREDENTIAL-MANAGEMENT.SV-OPERATIONAL-CREDENTIAL-CHECKPOINT-SERVICE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Workforce Qualification & Development Enablement → BEC CT3: Credential Management	Linked responsibility references; historical records do not assert current ancestry.
Evidence	hso:244 (p. 1462); hso:241 (p. 1462)	Source standing does not approve the derived assertion.

31.14.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical operational badges/physical-access credentials are distinct from reviewed clinical-service credential definitions.

31.14.2. Definition / governed semantic detail

Evaluates physical/operational access rights.

31.15. Function: Manage Specimen Transport Progression

Property	Value	Meaning
Reference Id	`HCM-FN-manage-specimen-transport-progression`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.FN-MANAGE-SPECIMEN-TRANSPORT-PROGRESSION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.FN-MANAGE-SPECIMEN-TRANSPORT-PROGRESSION`	Direct to this identity; never reusable.
Evidence	hso:267 (p. 1463)	Source standing does not approve the derived assertion.

31.15.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The Canonical Function includes pathology bio-specimens, blood products and surgical trays. Surgical-tray/materiel movement is excluded from reviewed Clinical Logistics Coordination; a specimen/materiel semantic split requires human judgement.

31.15.2. Definition / governed semantic detail

Tracks the physical transit of pathology bio-specimens, blood products, and surgical trays: *Collected → Courier Picked Up → In-Transit → Laboratory Ingress Received*.

31.16. Function: Mediate Standards-Based Health Information Communication

Property	Value	Meaning
Reference Id	`HCM-FN-mediate-standards-based-health-information-communication`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.

Property	Value	Meaning
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION.FN-MEDIATE-STANDARDS-BASED-HEALTH-INFORMATION-COMMUNICATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Connectivity Enablement → BEC CT3: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:97 (p. 1467)	Source standing does not approve the derived assertion.

31.16.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical boundary protocol mediation and technical transmission are distinct from reviewed patient-facing communication.

31.16.2. Definition / governed semantic detail

Handles standards-based communication mediation, transaction coordination, and technical error handling.

31.17. Service: Standards Communication Gateway Service

Property	Value	Meaning
Reference Id	`HCM-SV-standards-communication-gateway-service`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION.SV-STANDARDS-COMMUNICATION-GATEWAY-SERVICE`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Connectivity Enablement → BEC CT3: Health Information Communication	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:98 (p. 1467)	Source standing does not approve the derived assertion.

31.17.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical boundary protocol mediation and technical transmission are distinct from reviewed patient-facing communication.

31.17.2. Definition / governed semantic detail

Protocol interface for external client integrations.

31.18. Function: Coordinate Work Order

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-work-order`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-COORDINATE-WORK-ORDER`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:181 (p. 1469)	Source standing does not approve the derived assertion.

31.18.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.18.2. Definition / governed semantic detail

Coordinates operational task lifecycles and dispatches.

31.19. Service: Work Order Coordination Service

Property	Value	Meaning
Reference Id	`HCM-SV-work-order-coordination-service`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-WORK-ORDER-COORDINATION-SERVICE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:182 (p. 1469)	Source standing does not approve the derived assertion.

31.19.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.19.2. Definition / governed semantic detail

Manages operational work progression.

31.20. Function: Coordinate To Do

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-to-do`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-COORDINATE-TO-DO`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:185 (p. 1469)	Source standing does not approve the derived assertion.

31.20.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.20.2. Definition / governed semantic detail

Coordinates human review/update/approval items, reminders and sign-off queues within Harmonia's operational responsibility; clinical requirements and decisions remain with accountable clinical authority.

31.21. Service: To Do Management Service

Property	Value	Meaning
Reference Id	`HCM-SV-to-do-management-service`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-TO-DO-MANAGEMENT-SERVICE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:186 (p. 1469)	Source standing does not approve the derived assertion.

31.21.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.21.2. Definition / governed semantic detail

Provides To Do inbox and sign-off coordination without acquiring underlying clinical-work management.

31.22. Function: Coordinate Synthetic Task

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-synthetic-task`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-COORDINATE-SYNTHETIC-TASK`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:189 (p. 1469)	Source standing does not approve the derived assertion.

31.22.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.22.2. Definition / governed semantic detail

Orchestrates automated system workflows and operational progression.

31.23. Service: Synthetic Task Execution Service

Property	Value	Meaning
Reference Id	`HCM-SV-synthetic-task-execution-service`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-SYNTHETIC-TASK-EXECUTION-SERVICE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:190 (p. 1469)	Source standing does not approve the derived assertion.

31.23.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.23.2. Definition / governed semantic detail

Coordinates automated tasks.

31.24. Function: Supervise Activity Timeout / Escalation

Property	Value	Meaning
Reference Id	`HCM-FN-supervise-activity-timeout-escalation`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-SUPERVISE-ACTIVITY-TIMEOUT-ESCALATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:193 (p. 1469)	Source standing does not approve the derived assertion.

31.24.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.24.2. Definition / governed semantic detail

Monitors SLAs and triggers escalations when clinical or operational deadlines elapse.

31.25. Service: Activity Escalation Telemetry

Property	Value	Meaning
Reference Id	`HCM-SV-activity-escalation-telemetry`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-ACTIVITY-ESCALATION-TELEMETRY`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	ise:194 (p. 1469)	Source standing does not approve the derived assertion.

31.25.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.25.2. Definition / governed semantic detail

Discloses SLA breaches.

31.26. Function: Establish Assurance Context

Property	Value	Meaning
Reference Id	`HCM-FN-establish-assurance-context`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FN-ESTABLISH-ASSURANCE-CONTEXT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:98 (p. 1427)	Source standing does not approve the derived assertion.

31.26.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.26.2. Function Overview

Context establishment makes particular source-owned information evidence for a particular assurance purpose, with applicable subject, criteria and time identified. It includes evidence assembly without assuming sufficiency or prescribing copying and storage. This contextual evidentiary association is the assurance responsibility; ownership of the source material remains unchanged.

31.26.3. Definition / governed semantic detail

- **Canonical Name**: `EstablishAssuranceContext`.
- **Definition**: Establishes the governed subject, applicable assurance purpose and criteria, relevant temporal context, and contextual association of sufficient trustworthy source information as evidence required for an assurance activity.
- **Scope**: Includes the evidence-assembly responsibility previously described as Assurance Evidence Assembly; no separate evidence-assembly Function is established. Sufficiency is required for conclusion, not assumed merely because context is established; unavailable or insufficient evidence remains explicit.
- **Information Boundary**: Source information remains owned by its responsible source Capability. Its use as evidence does not transfer ownership or originating authority. Governed Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Evidence association and applicable criteria may each be temporal and version-specific; association distinguishes the information state relevant to assurance from its current state.
- **Realisation Boundary**: Uses generic EC-02 context-management capability together with EC-14 assurance-specific semantics as documented in §4.1. Establishment of Assurance Context prescribes no copying, persistence, aggregation, caching or other technical mechanism.

31.26.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.27. Function: Assess Assurance Evidence

Property	Value	Meaning
Reference Id	`HCM-FN-assess-assurance-evidence`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FN-ASSESS-ASSURANCE-EVIDENCE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:108 (p. 1427)	Source standing does not approve the derived assertion.

31.27.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.27.2. Function Overview

Evidence assessment applies the established evaluative basis to the available trustworthy material. It produces assessed detail rather than treating source availability or successful processing as a conclusion. Explicit gaps and qualifications preserve what the evidence cannot establish for subsequent adjudication.

31.27.3. Definition / governed semantic detail

- **Canonical Name**: `AssessAssuranceEvidence`.
- **Definition**: Evaluates the available trustworthy evidence against the applicable assurance criteria and establishes what that evidence demonstrates for each relevant criterion or assessment concern.
- **Boundary**: Assessment remains distinct from adjudication. It may identify satisfaction, exception, insufficiency or other criterion-level results where applicable; no universal result taxonomy is established. Quantitative, qualitative, compound, temporal and confidence-bearing assessment remain possible. **Insufficient evidence is not equivalent to either satisfaction or non-satisfaction.**

31.27.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.28. Function: Adjudicate Assurance Assessment

Property	Value	Meaning
Reference Id	`HCM-FN-adjudicate-assurance-assessment`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FN-ADJUDICATE-ASSURANCE-ASSESSMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:116 (p. 1428)	Source standing does not approve the derived assertion.

31.28.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.28.2. Function Overview

Adjudication establishes the supported finding or conclusion from assessed detail. It is distinct from gathering evidence and from reporting an outcome already reached. Insufficiency must remain explicit rather than be forced into satisfaction or non-satisfaction, and the resulting finding does not acquire authority to remediate the subject.

31.28.3. Definition / governed semantic detail

- **Canonical Name**: `AdjudicateAssuranceAssessment`.
- **Definition**: Determines the assurance finding or conclusion that follows from the assessed evidence and applicable assurance criteria.
- **Boundary**: Establishment of the finding/conclusion is part of adjudication. It remains distinct from evidence assessment, operational response, remediation, escalation, workflow progression and clinical judgement. No separate conclusion-establishment or reporting Function is introduced merely because a conclusion is established or may be communicated.

31.28.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.29. Service: Request Service Assurance

Property	Value	Meaning
Reference Id	`HCM-SV-request-service-assurance`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.SV-REQUEST-SERVICE-ASSURANCE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:164 (p. 1429)	Source standing does not approve the derived assertion.

31.29.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.29.2. Service Overview

This Service gives approved requesting roles a governed way to seek assurance within their authority. A request expresses the legitimate assurance need; it does not confer control over criteria, evidence assessment or adjudication. The requester can be involved in the subject while the required independence of the conclusion remains intact.

31.29.3. Definition / governed semantic detail

- **Canonical Name**: `RequestServiceAssurance`.
- **Definition**: Enables an authorised consumer to request that Service Assurance be performed for an applicable governed subject in accordance with an established Assurance Definition.
- **Approved Consumers**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Request, with Service Guardian and one authorised requesting Role.
- **Process Relationship**: Requests Assurance Process Execution under an applicable established Assurance Definition. It does not require a new Assurance Process Design instance for each request.
- **Independence Boundary**: The requesting Role identifies the assurance need within its responsibility and authority. Requesting does not grant authority to define or alter the Assurance Definition, alter Assurance Criteria, direct evidence assessment, determine adjudication or control the resulting Assurance Finding / Conclusion. REQ-FND-005 control independence remains applicable, including where a requester performs or manages the subject. Detailed payloads, contracts, initiation mechanics and states remain unresolved.

31.29.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.30. Service: Request Service Assurance Status

Property	Value	Meaning
Reference Id	`HCM-SV-request-service-assurance-status`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.SV-REQUEST-SERVICE-ASSURANCE-STATUS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:175 (p. 1430)	Source standing does not approve the derived assertion.

31.30.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.30.2. Service Overview

The status Service exposes progression of an identifiable assurance activity exclusively to System Steward. Its client question is where the activity has reached, not what the evidence will establish. Requesting assurance does not automatically grant status access, and progression observation is stewardship rather than recursive assurance.

31.30.3. Definition / governed semantic detail

- **Canonical Name**: `RequestServiceAssuranceStatus`.
- **Definition**: Enables the System Steward to obtain the current business progression state of an identifiable Service Assurance activity for stewardship purposes.
- **Only Approved Consumer**: `SystemSteward`.
- **Associated Interaction**: Service Assurance Status, between System Steward and Service Guardian.
- **Status Boundary**: Concerns **activity progression state**. It does not communicate provisional findings, incomplete assessment results as conclusions, predicted outcomes or unadjudicated conclusions. Other requesting Roles do not gain status access merely because they requested assurance. No detailed activity-status taxonomy is established.

Service Assurance Status answers "where is the assurance activity in its progression?" It does not answer "what has the assurance established?" The latter requires an

adjudicated Assurance Finding / Conclusion and subsequent outcome communication; neither status nor execution completion establishes that conclusion.

Management Monitoring evaluates activity state in order to progress or manage activity. Assurance evaluates activity against governing criteria in order to establish an assurance conclusion. System Steward's use of this Service is stewardship / management observation of assurance progression, not itself assurance or recursive assurance. The non-recursion boundary in §7.2 applies.

31.30.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.31. Service: Communicate Service Assurance Outcome

Property	Value	Meaning
Reference Id	`HCM-SV-communicate-service-assurance-outcome`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.SV-COMMUNICATE-SERVICE-ASSURANCE-OUTCOME`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	assurance:189 (p. 1430)	Source standing does not approve the derived assertion.

31.31.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.31.2. Service Overview

Outcome communication exposes already established findings to applicable authorised recipients from the approved roles. It preserves the adjudicated meaning and does not require a broadcast to every possible recipient. Receipt does not transfer the recipient's subsequent clinical, governance or operational action to Service Guardian.

31.31.3. Definition / governed semantic detail

- **Canonical Name**: `CommunicateServiceAssuranceOutcome`.
- **Definition**: Communicates applicable established Assurance Findings and Conclusions to authorised recipients according to their responsibilities, authority and information requirements.

- **Approved Potential Recipients**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Outcome Communication, with Service Guardian and one or more authorised recipient Roles.
- **Process Relationship**: Exposes the communication behaviour of Assurance Process Reporting / Communication, beginning from an already established finding/conclusion. It does not perform assessment or adjudication or alter/reinterpret the established outcome. A documentary report is not required.
- **Recipient Boundary**: No mandatory broadcast is established. A particular outcome may be communicated to one, some or all legitimate recipients according to applicability, authority, responsibility and information requirements. Requester, status consumer and outcome recipient need not be the same Role. Communication does not transfer responsibility for recipients' interpretation within their authority, operational/clinical management, remediation, escalation, policy/compliance response, process improvement or other subsequent action to Service Guardian.

31.31.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.32. Process: Specimen Transport Process

Property	Value	Meaning
Reference Id	`HCM-PR-specimen-transport-process`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.PR-SPECIMEN-TRANSPORT-PROCESS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Clinical Logistics Coordination	Linked responsibility references; historical records do not assert current ancestry.
Historical ID aliases	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.PR-SPECIMEN-TRANSPORT-PROCESS`	Direct to this identity; never reusable.
Evidence	processes:248 (p. 1419); processes:249 (p. 1419)	Source standing does not approve the derived assertion.

31.32.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The Canonical Process includes blood products as well as bio-specimens and surgical biopsies. Whether the blood-product wording denotes diagnostic specimens or transported health materiel is unadjudicated; the original Process is preserved without narrowing it.

31.32.2. Process Overview

Specimen transport preserves the movement and custody account connecting collection context with laboratory receipt. It makes the journey of the material explainable rather than reducing it to a sent/received pair. Delivery remains distinct from specimen acceptance, diagnostic performance and the eventual professional interpretation of its evidence.

31.32.3. Definition / governed semantic detail

- **Owning Capability**: `ClinicalLogisticsCoordination`
- **Process Purpose**: Governs the physical courier chain-of-custody, transport, and delivery of pathology bio-specimens, blood products, and surgical biopsies.
- **State Progression Lifecycle**: Specimen Packaged → Courier Dispatched → Chain-of-Custody Signed → In-Transit (Monitored) → Lab Ingress Received → Specimen Accepted
- **Key State Dispositions**:
  - `SpecimenPackaged`: Clinical unit packages bio-sample with barcode manifest.
  - `CourierDispatched`: Courier route assigned for pickup.
  - `Chain-of-CustodySigned`: Courier scans barcode and accepts legal custody.
  - `In-Transit(Monitored)`: Specimen in transit under temperature/time constraints.
  - `LabIngressReceived`: Central pathology specimen reception logs physical receipt.
  - `SpecimenAccepted`: Specimen integrity verified and passed to analytical track.

31.33. Process: Work Order Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-work-order-progression-process`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-WORK-ORDER-PROGRESSION-PROCESS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:282 (p. 1420); processes:283 (p. 1420)	Source standing does not approve the derived assertion.

31.33.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.33.2. Process Overview

Work Order progression describes a human undertaking physical or operational activity. Assignment, acknowledgement and outcome make the undertaking accountable without collapsing dispatch into completion. Its generic work meaning can support different operational contexts while leaving their domain authority and the clinical-work boundary intact.

31.33.3. Definition / governed semantic detail

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Doing** — physical or operational task executed by healthcare staff.
- **State Progression Lifecycle**: Created → Ready → Dispatched → Accepted → In-Progress → Completed / Failed / Cancelled
- **Key State Dispositions**:
  - `Created`: Work order instantiated with scope, target, and required skills.
  - `Ready`: Preconditions satisfied; available for assignment.
  - `Dispatched`: Sent to target individual or worker group queue.
  - `Accepted`: Worker claims task.
  - `In-Progress`: Execution underway.
  - `Completed`: Task finished with recorded completion telemetry.

31.34. Process: To Do Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-to-do-progression-process`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-TO-DO-PROGRESSION-PROCESS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:297 (p. 1421); processes:298 (p. 1421)	Source standing does not approve the derived assertion.

31.34.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.34.2. Process Overview

To Do progression coordinates a human review or decision item. Completion records progression of the item, while the significance and authority of the resulting judgement depend on the governed activity. A review being marked complete therefore does not independently establish clinical adequacy, signing authority or an assurance conclusion.

31.34.3. Definition / governed semantic detail

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Reviewing / Updating / Deciding** — clinical review, document countersignature, or administrative authorization task.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    I["Issued"] --> B["In-Inbox"]
    B --> R["Opened / Under-Review"]
    R --> A["Actioned / Decided"]
    D["Dismissed / Delegated<br/>Disposition/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>Relationship of dismissal/delegation
    ↪  to action/decision is unestablished."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,D note;
```

- **Key State Dispositions**:
  - `Issued`: To Do generated (e.g., review abnormal potassium result).
  - `In-Inbox`: Displayed in practitioner's actionable task list.
  - `Opened/Under-Review`: Clinician inspecting relevant clinical context.
  - `Actioned/Decided`: Decision executed (e.g., signed off, order placed, acknowledged).
  - `Dismissed/Delegated`: Task reassigned to registrar or dismissed with comment.
- **Preserved uncertainty**: Whether dismissal/delegation is an alternative disposition, a subsequent action or a scoped variant remains unresolved; its disconnected diagram node establishes no transition or position in the illustrative progression.

31.35. Process: Synthetic Task Progression Process

Property	Value	Meaning
Reference Id	`HCM-PR-synthetic-task-progression-process`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-SYNTHETIC-TASK-PROGRESSION-PROCESS`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:322 (p. 1421); processes:323 (p. 1421)	Source standing does not approve the derived assertion.

31.35.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.35.2. Process Overview

Synthetic Task progression concerns automated non-human execution, including waiting for a dependency and producing an output. It must remain distinct from human work orders and external clinical task lists. The disconnected stalled/failed outcome in the illustration leaves its relation to the success path unresolved; no reopening or post-completion failure transition is inferred.

31.35.3. Definition / governed semantic detail

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Non-Human Executable Work** — automated system workflows, batch syndications, or policy evaluation tasks.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    D["Draft / Scheduled"] --> E["Executing"]
    E --> W["Awaiting-Dependency"]
    W --> C["Completed"]
    F["Progression Stalled / Failed<br/>Outcome/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>No Completed-to-Stalled/Failed
    ↪  transition is established."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,F note;
```

- **Key State Dispositions**:
  - `Draft/Scheduled`: Automated task instantiated and scheduled for processing.
  - `Executing`: System actively processing task.
  - `Awaiting-Dependency`: Paused awaiting asynchronous external reply or event.
  - `Completed`: Automated execution succeeded and output artifact produced.
  - `ProgressionStalled/Failed`: Transient fault triggers scheduled recovery attempt; permanent failure records failure evidence.
- **Preserved uncertainty**: Whether stalled/failed is an alternative outcome, reopening/post-completion behaviour or a presentation defect remains unresolved. Its disconnected diagram node does not make success and failure equivalent or establish post-completion transitions.

31.36. Process: Assurance Process Execution

Property	Value	Meaning
Reference Id	`HCM-PR-assurance-process-execution`	Stable concept identity.

Property	Value	Meaning
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.PR-ASSURANCE-PROCESS-EXECUTION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:367 (p. 1422); processes:370 (p. 1422)	Source standing does not approve the derived assertion.

31.36.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.36.2. Process Overview

Assurance execution evaluates an identifiable subject under an applicable definition and establishes a finding through adjudication where sufficient evidence permits. Correct execution with insufficient evidence remains different from execution failure and from a positive or negative conclusion about the subject. Participating Functions describe responsibilities without supplying unapproved lifecycle states.

31.36.3. Definition / governed semantic detail

- **Canonical Name**: `AssuranceProcessExecution`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Performs defined assurance for an identifiable governed subject in accordance with an applicable Assurance Definition and establishes an Assurance Finding / Conclusion from applicable trustworthy evidence.
- **Principal Performing Role**: Service Guardian.
- **Participating Functions**: Establish Assurance Context, Assess Assurance Evidence and Adjudicate Assurance Assessment, all retaining Governed Assurance ownership.
- **Conceptual Basis and Output**: Applicable Assurance Definition plus identifiable governed subject; Assurance Finding / Conclusion established through adjudication where sufficient trustworthy evidence permits. Correct execution with insufficient evidence does not imply satisfaction or non-satisfaction or force a conclusion.
- **Boundary**: Function completion does not define Process lifecycle states. No Context Established, Evidence Assessed, Adjudicated or Concluded states are inferred. Detailed lifecycle states, transitions and initiation mechanics remain unresolved. Requesting this Process does not grant control of evidence assessment, adjudication or the resulting finding/conclusion.

RAD diagram: textual source notation.

```
graph TD
    DEF["Applicable Assurance Definition<br/>Conceptual basis"] -->|Assurance basis| EXEC["PR: Assurance Process
    ↪  Execution<br/>Owner: Governed Assurance"]
    SUBJECT["Identifiable Governed Subject"] -->|Subject of assurance| EXEC
    EXEC -->|Participating Function| CONTEXT["FN: Establish Assurance Context"]
    EXEC -->|Participating Function| ASSESS["FN: Assess Assurance Evidence"]
    EXEC -->|Participating Function| ADJ["FN: Adjudicate Assurance Assessment"]
    EXEC -->|Establishes through adjudication| FINDING["Assurance Finding / Conclusion"]
```

31.36.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.37. Process: Assurance Process Reporting / Communication

Property	Value	Meaning
Reference Id	`HCM-PR-assurance-process-reporting-communication`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Canonical	Canonical responsibility retained from v0.1.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Established	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.PR-ASSURANCE-PROCESS-REPORTING-COMMUNICATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	processes:389 (p. 1422); processes:392 (p. 1423)	Source standing does not approve the derived assertion.

31.37.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.37.2. Process Overview

This Process starts from an established finding and communicates it to legitimate recipients without altering the outcome. Communication need not create a report document, and it does not repeat assessment or adjudication. Recipients retain authority and responsibility for any clinical, operational, policy or improvement action they take in response.

31.37.3. Definition / governed semantic detail

- **Canonical Name**: `AssuranceProcessReporting/Communication`.
- **Owning Capability**: Governed Assurance.

- **Process Purpose**: Communicates applicable established Assurance Findings and Conclusions to legitimate recipients according to their responsibilities, authority and information requirements, without altering the established assurance outcome or assuming responsibility for the recipient's subsequent action.
- **Starting Basis**: An already established Assurance Finding / Conclusion. This Process does not perform assessment or adjudication, alter or reinterpret the established outcome.
- **Communication Participation**: Service Guardian participates through Service Assurance Outcome Communication, associated with Communicate Service Assurance Outcome. No additional Function is introduced to populate this Process.
- **Boundary**: Reporting / Communication is not limited to documentary report production; an appropriate Business Interaction may communicate the outcome without a report document. It assumes no operational response, remediation, escalation, policy/compliance response, process improvement, clinical management, clinical judgement or other recipient action. Detailed communication progression and completion semantics remain unresolved.

31.37.4. Preserved v0.1 Review Matters

No contextual-view placement is established.

31.38. BEC CT1: Care Context Integration Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-context-integration-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668	Source standing does not approve the derived assertion.

31.38.1. Capability Overview

Emergency presentation, inpatient rounds and outreach visits need different contextual assemblies of the same underlying information. This frame retains those established setting-specific responsibilities as compositions rather than duplicate records or clinical execution authorities.

31.38.2. Definition — Proposed

Compose governed information and coordination behaviour for materially different care settings, continuity arrangements and specialist clinical purposes.

31.38.3. Intent

Give a care participant the appropriate information context for the particular integration purpose without duplicating general information responsibilities.

31.38.4. Boundary

Setting, care mode and specialty are overlapping contextual dimensions. These children are integration responsibilities, not a mutually exclusive taxonomy of patients or clinical care.

31.38.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668

31.38.6. Reviewed successors

BEC CT1: Clinical Service Dependency Management (p. 491); BEC CT2: Patient Clinical Record (p. 656); BEC CT2: Episode & Encounter Administration (p. 363); BEC CT2: Emergency Department Operations (p. 548); BEC CT2: Ward Operations (p. 524); BEC CT2: Diagnostic Administration (p. 399); BEC CT2: Medication Administration (p. 417); BEC CT2: Client-Centred Administration (p. 328); BEC CT2: Care Plan Administration (p. 455)

31.39. BEC CT1: Human Collaboration & Work Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-human-collaboration-work-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT1	Type is independent of heading depth.
Direct placement	Proposed	Root placement.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668	Source standing does not approve the derived assertion.

31.39.1. Capability Overview

People discuss information, inspect it, make authorised decisions and undertake work. Those interactions need reusable coordination and temporal context, while entity-relative progression and activity design retain distinct semantics from message exchange and screen rendering.

31.39.2. Definition — Proposed

Enable contextual human interaction, collaboration and the definition and progression of governed operational activity.

31.39.3. Intent

Allow people and automated participants to interact and progress authorised work with intelligible context and accountability.

31.39.4. Boundary

Conversation is not a clinical record, presentation is not authority, and generic work coordination does not determine clinical work or own its outcome.

31.39.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10; architectural-axioms.md:668

31.39.6. Reviewed successors

BEC CT1: Shared Information & Coordination Enablement (p. 654)

31.40. BEC CT3: Provider Relationship

Property	Value	Meaning
Reference Id	`HCM-BEC-provider-relationship`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-RELATIONSHIP`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:150; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.40.1. Capability Overview

Effective affiliations explain which practitioner or team participates with which organisation. These relationships support collaboration and routing while each participating identity keeps its own responsibility owner.

31.40.2. Definition — Proposed

Managing organizational affiliations and multidisciplinary team relationships.

31.40.3. Intent

Preserve which affiliations apply when interpreting provider participation.

31.40.4. Boundary

Affiliation records do not transfer the participating organisations or practitioner identities to this capability.

31.40.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:150; information-responsibility.md:10

31.40.6. Reviewed successors

BEC CT3: Practitioner Role Administration (p. 249)

31.41. BEC CT3: Provider Engagement

Property	Value	Meaning
Reference Id	`HCM-BEC-provider-engagement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-ENGAGEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Provider Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:154; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.41.1. Capability Overview

Engagement information represents the terms and externally granted practice privileges under which a provider participates. This differs from who the practitioner is and from whether a particular operation is authorised now.

31.41.2. Definition — Proposed

Capturing terms of service and clinical privilege agreements.

31.41.3. Intent

Make the externally established terms and privilege standing of provider participation available without granting it.

31.41.4. Boundary

Records externally granted service terms and privileges; it does not independently grant those privileges.

31.41.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:154; information-responsibility.md:10

31.41.6. Reviewed successors

BEC CT3: Practitioner Role Administration (p. 249); BEC CT3: Provider Clinical Qualification Tracking (p. 623)

31.42. BEC CT3: Organisational Structure

Property	Value	Meaning
Reference Id	`HCM-BEC-organisational-structure`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATIONAL-STRUCTURE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:166; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.42.1. Capability Overview

Departments, divisions and regional networks need qualified structural relationships that can be traversed and understood over time. Structural membership is distinct from physical location and from authority to manage another organisation.

31.42.2. Definition — Proposed

Managing parent-child hierarchies, departments, and corporate relationships.

31.42.3. Intent

Make organisational relationships navigable without confusing structure with ownership or physical containment.

31.42.4. Boundary

Structural relationships do not acquire ownership of every related organisation or service.

31.42.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:166; information-responsibility.md:10

31.42.6. Reviewed successors

BEC CT3: Healthcare Organisation Profile Administration (p. 265)

31.43. BEC CT3: Organisation Contact

Property	Value	Meaning
Reference Id	`HCM-BEC-organisation-contact`	Stable concept identity.

Property	Value	Meaning
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATION-CONTACT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Organisation Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:170; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.43.1. Capability Overview

Verified facility contact channels make an organisation reachable for appropriate administrative and clinical communication. The contact record says how it may be reached; recipient qualification and disclosure policy govern a particular use.

31.43.2. Definition — Proposed

Maintaining verified administrative and clinical communication channels.

31.43.3. Intent

Provide verified channels for reaching the intended healthcare organisation.

31.43.4. Boundary

Contact validity is distinct from clinical recipient eligibility and policy permission.

31.43.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:170; information-responsibility.md:10

31.43.6. Reviewed successors

BEC CT3: Healthcare Organisation Profile Administration (p. 265)

31.44. BEC CT3: Location Structure

Property	Value	Meaning
Reference Id	`HCM-BEC-location-structure`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-LOCATION-STRUCTURE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Location Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:181; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.44.1. Capability Overview

Campus, building, floor, ward and room relationships explain physical containment. The structure provides navigation and context without determining organisational reporting lines or current operational capacity.

31.44.2. Definition — Proposed

Modeling the physical architecture of healthcare campuses.

31.44.3. Intent

Allow a particular place to be interpreted within its physical surroundings.

31.44.4. Boundary

Physical containment differs from organisational structure and operational responsibility.

31.44.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:181; information-responsibility.md:10

31.44.6. Reviewed successors

BEC CT3: Healthcare Location Profile Administration (p. 277)

31.45. BEC CT3: Service Provision Relationship

Property	Value	Meaning
Reference Id	`HCM-BEC-service-provision-relationship`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-PROVISION-RELATIONSHIP`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:196; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.45.1. Capability Overview

Service provision joins a named offering to the organisations, places and practitioner roles through which it is provided. The mapping is the owned concern; the identity and meaning of each mapped participant remain with its established capability.

31.45.2. Definition — Proposed

Binding health services to specific organizations, locations, and practitioner roles.

31.45.3. Intent

Make it clear which provider delivers which service at which place and through which participating roles.

31.45.4. Boundary

Owns mapping associations, retaining the owners of each participating entity.

31.45.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:196; information-responsibility.md:10

31.45.6. Reviewed successors

BEC CT3: Healthcare Service Profile Administration (p. 289)

31.46. BEC CT3: Service Availability Definition

Property	Value	Meaning
Reference Id	`HCM-BEC-service-availability-definition`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-AVAILABILITY-DEFINITION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:200; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.46.1. Capability Overview

Regular hours, on-call windows and exceptional closures describe when an offering is nominally available. They provide an input to readiness and scheduling without proving that all required dependencies are ready.

31.46.2. Definition — Proposed

Modeling regular operating hours, exceptional closures, and emergency availability.

31.46.3. Intent

Express the operating windows against which a service participation request can be considered.

31.46.4. Boundary

Represents operating windows and exceptions; it does not establish staffing, capacity or readiness.

31.46.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:200; information-responsibility.md:10

31.46.6. Reviewed successors

BEC CT3: Healthcare Service Profile Administration (p. 289)

31.47. BEC CT3: Service Eligibility Definition

Property	Value	Meaning
Reference Id	`HCM-BEC-service-eligibility-definition`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-ELIGIBILITY-DEFINITION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Health Service Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:204; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.47.1. Capability Overview

Published catchment, referral and clinical eligibility conditions explain the intended audience and access constraints of an offering. Their representation supports responsible assessment without deciding acceptance or clinical priority.

31.47.2. Definition — Proposed

Capturing referral criteria, catchment boundaries, and clinical eligibility.

31.47.3. Intent

Make the governed intake and catchment conditions available to the party responsible for access decisions.

31.47.4. Boundary

Represents governed eligibility criteria; it does not make clinical acceptance or triage decisions.

31.47.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:204; information-responsibility.md:10

31.47.6. Reviewed successors

BEC CT3: Healthcare Service Profile Administration (p. 289)

31.48. BEC CT3: Device Instance

Property	Value	Meaning
Reference Id	`HCM-BEC-device-instance`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-INSTANCE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:222; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.48.1. Capability Overview

An identifiable physical device carries UDI and serial context through its use. The instance record supports association and resolution without originating device observations or making maintenance decisions.

31.48.2. Definition — Proposed

Tracking physical device hardware instances and serial numbers.

31.48.3. Intent

Keep the identity of the actual physical equipment distinguishable from its model and endpoint.

31.48.4. Boundary

Identifies physical hardware; serial tracking is distinct from maintenance and live observation content.

31.48.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:222; information-responsibility.md:10

31.48.6. Reviewed successors

BEC CT3: Clinical Device Profile Administration (p. 307)

31.49. BEC CT3: Device Association

Property	Value	Meaning
Reference Id	`HCM-BEC-device-association`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-ASSOCIATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:226; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.49.1. Capability Overview

A device can be associated with different patients and care places at different times. Qualified association and its lineage allow observations to be interpreted against the applicable binding rather than the current assignment alone.

31.49.2. Definition — Proposed

Associating clinical devices with patients, beds, or rooms.

31.49.3. Intent

Allow a device-originated item to be associated with the correct subject and place at the relevant time.

31.49.4. Boundary

Represents qualified device-to-subject/place association without owning those subjects or observations.

31.49.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:226; information-responsibility.md:10

31.49.6. Reviewed successors

BEC CT3: Device Utilisation Administration (p. 311); BEC CT3: Device OA&M Metadata Management (p. 316)

31.50. BEC CT3: Device Connectivity

Property	Value	Meaning
Reference Id	`HCM-BEC-device-connectivity`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-CONNECTIVITY`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Entity Information Enablement → BEC CT2: Clinical Device Administration	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-enabling-capabilities.md:230; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.50.1. Capability Overview

A device communication binding identifies the protocol and endpoint through which its feed can participate. Connectivity metadata supports routing without owning authentication issuance, payload semantics or physiological interpretation.

31.50.2. Definition — Proposed

Registering communication protocols and network endpoints for medical devices.

31.50.3. Intent

Make a device feed reachable through its registered communication binding.

31.50.4. Boundary

Represents communication bindings; it does not acquire authentication credential issuance or telemetry interpretation.

31.50.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-enabling-capabilities.md:230; information-responsibility.md:10

31.50.6. Reviewed successors

BEC CT3: Device OA&M Metadata Management (p. 316)

31.51. BEC CT2: Care Access Transaction Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-access-transaction-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.51.1. Capability Overview

Coordinate the intake, care-request disposition and appointment information through which a person accesses care. Its constituent abilities distinguish Client-Centred Administration; Referral Administration; Scheduling Administration.

31.51.2. Definition — Proposed

Coordinate the intake, care-request disposition and appointment information through which a person accesses care.

31.51.3. Intent

Make an access request and its disposition traceable without confusing a booking with care responsibility or delivery.

31.51.4. Boundary

A referral requests care responsibility; an appointment concerns a booking. Neither establishes care delivery or host booking authority.

31.51.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.51.6. Reviewed successors

BEC CT2: Client-Centred Administration (p. 328); BEC CT2: Referral Administration (p. 342); BEC CT2: Scheduling Administration (p. 355)

31.52. BEC CT2: Care Fulfilment Administration Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-fulfilment-administration-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.52.1. Capability Overview

Maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. Its constituent abilities distinguish Episode & Encounter Administration; Order Administration; Diagnostic Administration; Medication Administration; Procedure Administration.

31.52.2. Definition — Proposed

Maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems.

31.52.3. Intent

Maintain the link between a bounded care request, the relevant episode and the externally observed fulfilment outcome.

31.52.4. Boundary

Encounter progression, order outcome, diagnostic interpretation, medication events and procedure documentation retain different source authorities.

31.52.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49;

information-responsibility.md:10

31.52.6. Reviewed successors

BEC CT2: Episode & Encounter Administration (p. 363); BEC CT2: Order Administration (p. 383); BEC CT2: Diagnostic Administration (p. 399); BEC CT2: Medication Administration (p. 417); BEC CT2: Procedure Administration (p. 435)

31.53. BEC CT2: Care Documentation & Communication Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-documentation-communication-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.53.1. Capability Overview

Administer the lifecycle, shared-plan context and directed communication of care documentation. Its constituent abilities distinguish Clinical Record Administration; Care Plan Administration; Clinical Communication Administration.

31.53.2. Definition — Proposed

Administer the lifecycle, shared-plan context and directed communication of care documentation.

31.53.3. Intent

Preserve which document or plan applies and what a directed recipient has actually acknowledged.

31.53.4. Boundary

This frame does not absorb longitudinal synthesis, collaboration discourse, originating clinical decisions or generic transport.

31.53.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.53.6. Reviewed successors

BEC CT2: Clinical Record Administration (p. 443); BEC CT2: Care Plan Administration (p. 455); BEC CT2: Clinical Communication Administration (p. 476)

31.54. BEC CT2: Care Financial Information Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-financial-information-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FINANCIAL-INFORMATION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Administration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.54.1. Capability Overview

Represent coverage context and convey care-activity evidence to responsible funding and charging systems. Its constituent abilities distinguish Funding & Coverage Administration; Clinical Charging & Claims Administration.

31.54.2. Definition — Proposed

Represent coverage context and convey care-activity evidence to responsible funding and charging systems.

31.54.3. Intent

Give responsible financial systems the care-activity and coverage context they need without creating another financial authority.

31.54.4. Boundary

Adjacent information participation does not provide financial governance, coverage adjudication or a billing platform.

31.54.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.54.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.54.7. Reviewed successors

BEC CT2: Funding & Coverage Administration (p. 487); BEC CT2: Clinical Charging & Claims Administration (p. 489)

31.55. BEC CT2: Longitudinal & Community Care Integration

Property	Value	Meaning
Reference Id	`HCM-BEC-longitudinal-community-care-integration`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Context Integration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.55.1. Capability Overview

Support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. Its constituent abilities distinguish Primary Care; Community Care; Outreach Care; Remote & Virtual Care; Preventive Care.

31.55.2. Definition — Proposed

Support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context.

31.55.3. Intent

Support continuity when care information must follow the person across providers, visits and distance.

31.55.4. Boundary

Longitudinal purpose does not make these care contexts exclusive. Outreach location and remote mode remain distinct from source clinical responsibility.

31.55.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.55.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.55.7. Reviewed successors

BEC CT2: Patient Clinical Record (p. 656); BEC CT2: Care Plan Administration (p. 455); BEC CT2: Client-Centred Administration (p. 328)

31.56. BEC CT2: Facility & Episodic Care Integration

Property	Value	Meaning
Reference Id	`HCM-BEC-facility-episodic-care-integration`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Last structural ID — historical	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Context Integration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.56.1. Capability Overview

Provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. Its constituent abilities distinguish Acute Care; Emergency Care; Inpatient Care; Ambulatory Care; Surgical & Procedural Care.

31.56.2. Definition — Proposed

Provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes.

31.56.3. Intent

Make presentation- and episode-specific information available to the responsible care team in its applicable context.

31.56.4. Boundary

Acute, emergency and inpatient perspectives overlap; each capability owns its named integration purpose rather than a population or clinical workflow.

31.56.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.56.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.56.7. Reviewed successors

BEC CT2: Episode & Encounter Administration (p. 363); BEC CT2: Emergency Department Operations (p. 548); BEC CT2: Ward Operations (p. 524); BEC CT1: Clinical Service Dependency Management (p. 491)

31.57. BEC CT2: Specialist Clinical Context Integration

Property	Value	Meaning
Reference Id	`HCM-BEC-specialist-clinical-context-integration`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Care Context Integration Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.57.1. Capability Overview

Integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. Its constituent abilities distinguish Clinical Assessment; Diagnostic Services; Therapeutic Services; Medication Therapy; Allied Health; Rehabilitation.

31.57.2. Definition — Proposed

Integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context.

31.57.3. Intent

Keep specialised clinical context connected to the wider record while retaining the specialist source of clinical judgement.

31.57.4. Boundary

A clinical specialty is evidence for integration purpose, not authority to invent a specialist engine, clinical decision or duplicated general capture function.

31.57.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.57.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.57.7. Reviewed successors

BEC CT2: Diagnostic Administration (p. 399); BEC CT2: Medication Administration (p. 417); BEC CT1: Clinical Service Dependency Management (p. 491)

31.58. BEC CT2: Care Setting Operations Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-care-setting-operations-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.58.1. Capability Overview

Coordinate the local flow, session state and capacity semantics of a care-delivery setting. Its constituent abilities distinguish Clinic & Practice Operations; Ward Operations; Theatre Operations; Emergency Department Operations; Outpatient Operations.

31.58.2. Definition — Proposed

Coordinate the local flow, session state and capacity semantics of a care-delivery setting.

31.58.3. Intent

Express the actual operating constraints of each setting rather than substituting a generic resource count for capacity.

31.58.4. Boundary

Local capacity is established here; aggregate telemetry, bed state and equipment state cannot substitute for it.

31.58.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.58.6. Reviewed successors

BEC CT2: Clinic & Practice Operations (p. 514); BEC CT2: Ward Operations (p. 524); BEC CT2: Theatre Operations (p. 538); BEC CT2: Emergency Department Operations (p. 548); BEC CT2: Outpatient Operations (p. 561)

31.59. BEC CT2: Care Resource & Capacity Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-care-resource-capacity-coordination`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.59.1. Capability Overview

Relate operational care-place and equipment state to the coordinated view of service demand and capacity. Its constituent abilities distinguish Bed & Care-Place Management; Clinical Resource Management; Service Capacity Management.

31.59.2. Definition — Proposed

Relate operational care-place and equipment state to the coordinated view of service demand and capacity.

31.59.3. Intent

Make resource-state contributions and coordinated capacity views usable without collapsing their different responsibilities.

31.59.4. Boundary

Bed/equipment availability is a contribution to capacity, not its definition. Higher-order capacity consumes local capacity without taking its ownership.

31.59.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.59.6. Reviewed successors

BEC CT2: Bed & Care-Place Management (p. 569); BEC CT2: Service Capacity Management (p. 581); BEC CT3: Device Utilisation Administration (p. 311); BEC CT1: Clinical Service Dependency Management (p. 491)

31.60. BEC CT2: Operational Workforce Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-operational-workforce-coordination`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.60.1. Capability Overview

Make workforce availability, coverage and eligible operational work participation usable in coordination. Its constituent abilities distinguish Clinical Workforce Management; Clinical Rostering; Mobile Staff Management; On-Call Management; Work Allocation & Dispatch.

31.60.2. Definition — Proposed

Make workforce availability, coverage and eligible operational work participation usable in co-ordination.

31.60.3. Intent

Support eligible operational participation using qualified availability and coverage information.

31.60.4. Boundary

Source rosters remain separately authoritative; operational dispatch does not acquire clinical-work allocation authority.

31.60.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.60.6. Reviewed successors

BEC CT2: Clinical Workforce Capability Monitoring (p. 587); BEC CT2: Clinical Rostering (p. 589); BEC CT2: Mobile Staff Management (p. 591); BEC CT2: On-Call Management (p. 601); BEC CT2: Work Allocation & Dispatch (p. 607)

31.61. BEC CT2: Workforce Qualification & Development Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-workforce-qualification-development-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.61.1. Capability Overview

Represent learning, competence, professional-standing checks and separately governed operational access qualifications. Its constituent abilities distinguish Clinical Training & Education; Clinical Competency Management; Clinical Qualification Management; Clinical Supervision; Credential Management.

31.61.2. Definition — Proposed

Represent learning, competence, professional-standing checks and separately governed operational access qualifications.

31.61.3. Intent

Keep professional standing, competence, supervision and operational-access evidence distinguishable when work is considered.

31.61.4. Boundary

Clinical qualifications and physical badges remain distinct CT3 responsibilities. Representation or checking does not grant privilege or become an authentication provider.

31.61.5. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.61.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.61.7. Reviewed successors

BEC CT2: Clinical Training & Education (p. 618); BEC CT2: Clinical Competency Administration (p. 619); BEC CT2: Clinical Qualification Administration (p. 621); BEC CT2: Clinical Supervision (p. 630); BEC CT2: Credential Management (p. 627)

31.62. BEC CT2: Care Movement & Transition Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-care-movement-transition-coordination`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.62.1. Capability Overview

Coordinate movement of people/materials and operational departure readiness across care boundaries. Its constituent abilities distinguish Patient Transport; Clinical Logistics Coordination; Discharge Management.

31.62.2. Definition — Proposed

Coordinate movement of people/materials and operational departure readiness across care boundaries.

31.62.3. Intent

Coordinate the operational prerequisites and observed milestones of movement and departure without assuming clinical clearance.

31.62.4. Boundary

Physical movement, clinical handover, discharge clinical authorisation and care-responsibility transition remain distinct.

31.62.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.62.6. Reviewed successors

BEC CT2: Patient Transport (p. 632); BEC CT2: Clinical Logistics Coordination (p. 639); BEC CT2: Discharge Management (p. 645)

31.63. BEC CT2: Clinical Information Stewardship Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-clinical-information-stewardship-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.63.1. Capability Overview

Maintain governed longitudinal synthesis and the associations needed to use its constituent clinical information safely. Its constituent abilities distinguish Patient Clinical Record.

31.63.2. Definition — Proposed

Maintain governed longitudinal synthesis and the associations needed to use its constituent clinical information safely.

31.63.3. Intent

Make the integrated clinical account usable while keeping its source assertions and associated content attributable.

31.63.4. Boundary

The synthesis preserves what Harmonia received or managed without becoming the originator of the clinical facts.

31.63.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.63.6. Reviewed successors

BEC CT2: Patient Clinical Record (p. 656)

31.64. BEC CT2: Health Information Connectivity Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-health-information-connectivity-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.64.1. Capability Overview

Make governed health information discoverable and exchangeable under explicit internal and external contracts. Its constituent abilities distinguish Health Information Exchange; Health Information Access; Health Information Communication.

31.64.2. Definition — Proposed

Make governed health information discoverable and exchangeable under explicit internal and external contracts.

31.64.3. Intent

Let authorised participants find and exchange the information their contract permits.

31.64.4. Boundary

Query execution, transaction coordination and boundary protocol mediation are separate responsibilities; none transfers content ownership.

31.64.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.64.6. Reviewed successors

BEC CT2: Health Information Exchange Enablement (p. 686); BEC CT2: Health Information Access (p. 701); BEC CT2: Health Information Communication (p. 710)

31.65. BEC CT2: Semantic & Reference Knowledge Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-semantic-reference-knowledge-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.65.1. Capability Overview

Govern the definitions, terminology and reference content that make healthcare information consistently interpretable. Its constituent abilities distinguish Clinical Knowledge Services; Information Design Governance.

31.65.2. Definition — Proposed

Govern the definitions, terminology and reference content that make healthcare information consistently interpretable.

31.65.3. Intent

Keep structural meaning, terminology bindings and approved reference content consistent enough for shared interpretation.

31.65.4. Boundary

Definition semantics, mutable approved content and runtime policy decisions retain distinct ownership.

31.65.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.65.6. Reviewed successors

BEC CT2: Clinical Knowledge Services (p. 742); BEC CT2: Information Design Governance (p. 792)

31.66. BEC CT2: Information Protection & Accountability Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-information-protection-accountability-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.66.1. Capability Overview

Apply information-use policy and preserve the accountability context needed for governed health-information activity. Its constituent abilities distinguish Health Information Control.

31.66.2. Definition — Proposed

Apply information-use policy and preserve the accountability context needed for governed health-information activity.

31.66.3. Intent

Make information use both governed and explainable through policy, attributable rules and significant evidence.

31.66.4. Boundary

Patient directives remain with Client Privacy; runtime authorisation does not become external authentication or clinical authority.

31.66.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.66.6. Reviewed successors

BEC CT2: Health Information Control (p. 715); BEC CT2: Information Content Maintainer Management (p. 731)

31.67. BEC CT2: Collaborative Interaction Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-collaborative-interaction-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.67.1. Capability Overview

Support contextual discussion and human interaction with governed information and activity. Its constituent abilities distinguish Clinical Collaboration; General Collaboration; Presentation Services.

31.67.2. Definition — Proposed

Support contextual discussion and human interaction with governed information and activity.

31.67.3. Intent

Give participants a useful discussion and interaction context without turning the communication medium into a clinical authority.

31.67.4. Boundary

Collaboration control, collaboration operations and general presentation remain distinct; a discussion or screen is not a new clinical fact.

31.67.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.67.6. Reviewed successors

BEC CT2: Clinical Collaboration (p. 756); BEC CT2: General Collaboration (p. 768); BEC CT2: Presentation Services (p. 786)

31.68. BEC CT2: Operational Activity & Temporal Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-operational-activity-temporal-enablement`	Stable concept identity.
Reconciliation standing	superseded	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.68.1. Capability Overview

Progress governed operational work in identifiable activity and temporal contexts. Its constituent abilities distinguish Workflow & Activity Coordination; Calendar Management.

31.68.2. Definition — Proposed

Progress governed operational work in identifiable activity and temporal contexts.

31.68.3. Intent

Keep work progression and its timing intelligible across human and synthetic undertakings.

31.68.4. Boundary

Work Order is human doing, To Do is human review/update/approval and Task is synthetic work. Projection does not own authoritative bookings or rosters.

31.68.5. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.68.6. Reviewed successors

BEC CT2: Workflow & Activity Coordination (p. 773); BEC CT2: Calendar Management (p. 779)

31.69. BEC CT2: Independent Assurance Evaluation Enablement

Property	Value	Meaning
Reference Id	`HCM-BEC-independent-assurance-evaluation-enablement`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT2	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-architecture-metamodel.md:49; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.69.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The previous grouping frames Canonical subject-assurance execution; it cannot be equated with review of the assurance service itself.

31.69.2. Capability Overview

Associate trustworthy evidence, assess it and adjudicate independently reportable findings about governed service behaviour. Its constituent abilities distinguish Governed Assurance.

31.69.3. Definition — Proposed

Associate trustworthy evidence, assess it and adjudicate independently reportable findings about governed service behaviour.

31.69.4. Intent

Produce findings supported by applicable evidence and criteria while retaining inability to conclude.

31.69.5. Boundary

The subject can supply evidence but cannot control its conclusion. Evidence insufficiency remains distinct from execution uncertainty and clinical judgement.

31.69.6. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-architecture-metamodel.md:49; information-responsibility.md:10

31.70. BEC CT3: Information Authority Representation

Property	Value	Meaning
Reference Id	`HCM-BEC-information-authority-representation`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-INFORMATION-AUTHORITY-REPRESENTATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Information Stewardship Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	architectural-axioms.md:303; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.70.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Source credibility and authority standing require an explicit home; information-asset tags alone do not establish that responsibility.

31.70.2. Capability Overview

A persisted assertion may be authoritative, informational or anecdotal. This ability carries that standing into interpretation without treating transport reliability, freshness or successful commit as proof of credibility.

31.70.3. Definition — Proposed

Represent the source, credibility and qualified authority standing of competing health-information assertions.

31.70.4. Intent

Keep an assertion technical acceptance separate from the standing of the source that asserted it.

31.70.5. Boundary

Classification does not originate clinical facts or universally adjudicate conflicts. Policy determines how qualified assertions are used.

31.70.6. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.70.7. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:303; information-responsibility.md:10

31.71. BEC CT3: Significant Activity Audit Capture

Property	Value	Meaning
Reference Id	`HCM-BEC-significant-activity-audit-capture`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Protection & Accountability Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	architectural-axioms.md:379; information-responsibility.md:10	Source standing does not approve the derived assertion.

31.71.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The reviewed hierarchy does not allocate general mandatory authoritative audit capture; disclosure or provenance alone is not an equivalent responsibility.

31.71.2. Capability Overview

Mandatory evidence capture has its own policy-governed platform authority and must remain possible when the audited identity is denied an action. The captured fact is distinguished from debug output and the identity whose activity is recorded.

31.71.3. Definition — Proposed

Capture required security- and information-significant activity as governed, attributable audit evidence.

31.71.4. Intent

Ensure required significant activity evidence can be captured under the authority specifically assigned to that responsibility.

31.71.5. Boundary

The specific intrinsic authority for mandatory audit capture is not generalised to other activities. Capture is not an assurance finding and cannot expose unmasked PHI in diagnostic logs.

31.71.6. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.71.7. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:379; information-responsibility.md:10

31.72. BEC CT3: Governance Requirement Publication Support

Property	Value	Meaning
Reference Id	`HCM-BEC-governance-requirement-publication-support`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-GOVERNANCE-REQUIREMENT-PUBLICATION-SUPPORT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Protection & Accountability Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:993; health-service-assurance.md:9	Source standing does not approve the derived assertion.

31.72.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Requirement publication/authority is not assurance-service review or information-model design; the reviewed hierarchy does not allocate a home.

31.72.2. Capability Overview

Consumers need an identifiable statement of which rule applies and who established it. This proposal fills BC-17 information support without giving the platform authority to make legislation, approve governance requirements or reinterpret their force.

31.72.3. Definition — Proposed

Represent and publish versioned governing requirements for use by authorised operational and assurance consumers.

31.72.4. Intent

Let operational and assurance consumers identify the governing statement and applicability basis they actually used.

31.72.5. Boundary

Governance approval remains externally mandated; operational policy, assurance criteria and requirement authority are not interchangeable.

31.72.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

31.72.7. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:993; health-service-assurance.md:9

31.73. BEC CT3: Entity-Specific Operational Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-entity-specific-operational-coordination`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73; architectural-axioms.md:742	Source standing does not approve the derived assertion.

31.73.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal entity-relative progression is not the reviewed external work bridge; a current BEC destination has not been adjudicated.

31.73.2. Capability Overview

Activity concerning the same patient, practitioner or bed needs a coherent entity-relative frame. Interest fan-out, serial progression, fresh information context and uncertain effects remain distinguishable from generic multi-work-unit progression.

31.73.3. Definition — Proposed

Coordinate governed operational progression relative to one represented entity, including interested behaviour participation and controlled recovery.

31.73.4. Intent

Keep the progression of one represented entity coherent across interested operational behaviours.

31.73.5. Boundary

Entity coordination does not own all entity information, determine clinical work or become the source of clinical decisions. No Twin component allocation follows from this proposal.

31.73.6. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.73.7. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73; architectural-axioms.md:742

31.74. BEC CT3: Governed Operational Activity Design

Property	Value	Meaning
Reference Id	`HCM-BEC-governed-operational-activity-design`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	context-and-persistence-management.md:52; pathways-journeys-and-handoff.md:37	Source standing does not approve the derived assertion.

31.74.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal execution-envelope design is distinct from governance of work across an external execution boundary.

31.74.2. Capability Overview

An activity needs a fulfilment definition even when only one bounded execution unit is involved. This capability specifies its semantic envelope while execution and event delivery remain separate responsibilities.

31.74.3. Definition — Proposed

Define the information context, expected outcomes, continuation conditions and housekeeping of governed operational activity.

31.74.4. Intent

Make the expected information and fulfilment basis of an undertaking explicit before it executes.

31.74.5. Boundary

An activity definition is neither an orchestrator nor a choreographer. It does not choose clinical care or dynamically invent execution logic.

31.74.6. Review Confidence

Medium; applies to the proposal, not to preserved canonical meaning.

31.74.7. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task context-and-persistence-management.md:52; pathways-journeys-and-handoff.md:37

31.75. BEC CT3: Service Offering Readiness Coordination

Property	Value	Meaning
Reference Id	`HCM-BEC-service-offering-readiness-coordination`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	capability / CT3	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Care Resource & Capacity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Harmonia relevance	Proposed materiality; no inherited Core classification	Independent of element/placement status; never inherited from an ancestor.
Evidence	business-capabilities.md:403; information-responsibility.md:34	Source standing does not approve the derived assertion.

31.75.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: source proposal determines collective readiness; reviewed scope establishes dependency tracking only. Evidence may be reused, but determination authority is not silently transferred.

31.75.2. Capability Overview

Collective readiness depends on suitable workforce, spaces, equipment, materials and supporting services. This proposal evaluates their joint availability for an offering without treating one available bed, an operating-hours declaration or aggregate capacity as service readiness.

31.75.3. Definition — Proposed

Assemble the dependency evidence needed to determine and communicate whether a defined service offering is operationally ready in a particular context.

31.75.4. Intent

Distinguish collective service deliverability from nominal availability or readiness of any single dependency.

31.75.5. Boundary

Contributing capabilities retain source readiness and resource management. Clinical go/no-go decisions and service governance are not granted by this information-based coordination.

31.75.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

31.75.7. Derivation Provenance

Inferred from bounded responsibilities and their semantic decomposition. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:403; information-responsibility.md:34

31.75.8. Reviewed successors

BEC CT1: Clinical Service Dependency Management (p. 491)

31.76. Feature: Information Authority & Source Standing Representation

Property	Value	Meaning
Reference Id	`HCM-FT-information-authority-source-standing-representation`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-INFORMATION-AUTHORITY-REPRESENTATION.FEAT-STEP2-INFORMATION-AUTHORITY-SOURCE-STANDING-REPRESENTATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Information Stewardship Enablement → BEC CT3: Information Authority Representation	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:303	Source standing does not approve the derived assertion.

31.76.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Source credibility and authority standing require an explicit home; information-asset tags alone do not establish that responsibility.

31.76.2. Feature Overview

The user of an assertion needs to distinguish authoritative, informational and anecdotal standing even when each was technically valid and durably accepted.

31.76.3. Definition — Proposed

Attach and preserve qualified source-authority and credibility standing for managed assertions.

31.76.4. Boundary

Authority classification is not a clinical truth judgement or a universal rule for choosing competing assertions.

31.76.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Information Authority Representation, rather than inheriting an adjacent responsibility.

31.76.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:303

31.77. Feature: Mandatory Significant Activity Audit Capture

Property	Value	Meaning
Reference Id	`HCM-FT-mandatory-significant-activity-audit-capture`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE.FEAT-STEP2-MANDATORY-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Protection & Accountability Enablement → BEC CT3: Significant Activity Audit Capture	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:379	Source standing does not approve the derived assertion.

31.77.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The reviewed hierarchy does not allocate general mandatory authoritative audit capture; disclosure or provenance alone is not an equivalent responsibility.

31.77.2. Feature Overview

The actor whose action is recorded need not be entitled to persist the audit. Required capture uses its own established authority and failure can block the governed operation.

31.77.3. Definition — Proposed

Capture required significant activity evidence using the specifically governed authoritative audit-capture boundary.

31.77.4. Boundary

Specific audit authority is not borrowed from the audited identity or generalised to other operations.

31.77.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Significant Activity Audit Capture, rather than inheriting an adjacent responsibility.

31.77.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:379

31.77.7. Prior Candidate Derivation

v0.3 reconciliation: `BEC-7D40E369` (line 2963); `BEC-1D2CA7D5` (line 9092); `EC-829F0BF7` (line 10569). Working locators are not identity aliases.

31.78. Feature: Trusted Identity Context Intake

Property	Value	Meaning
Reference Id	`HCM-FT-trusted-identity-context-intake`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.

Property	Value	Meaning
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-STEP2-TRUSTED-IDENTITY-CONTEXT-INTAKE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33	Source standing does not approve the derived assertion.

31.78.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: general platform security/evidence Feature has no adjudicated patient-specific CT3 frame; established Health Information Control responsibility is preserved.

31.78.2. Feature Overview

The boundary between WHO and WHAT remains explicit: an external identity assertion is verified as trusted input before internal authorisation is resolved.

31.78.3. Definition — Proposed

Accept and qualify externally authenticated identity context for Harmonia-governed operations.

31.78.4. Boundary

Harmonia does not become an authentication or credential-issuance provider.

31.78.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Health Information Control, rather than inheriting an adjacent responsibility.

31.78.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33

31.79. Feature: Governed Policy Assignment & Revocation

Property	Value	Meaning
Reference Id	`HCM-FT-governed-policy-assignment-revocation`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-STEP2-GOVERNED-POLICY-ASSIGNMENT-REVOCATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33	Source standing does not approve the derived assertion.

31.79.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: general platform security/evidence Feature has no adjudicated patient-specific CT3 frame; established Health Information Control responsibility is preserved.

31.79.2. Feature Overview

An assignment becoming inapplicable by expiry differs from an explicit revocation; both must affect the effective authority used for operations.

31.79.3. Definition — Proposed

Manage approved permission criteria and identity/role assignments, including temporal applicability and revocation.

31.79.4. Boundary

Management does not grant unconstrained administrative authority or derive permission from resource tags.

31.79.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Health Information Control, rather than inheriting an adjacent responsibility.

31.79.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33

31.80. Feature: Operation-Bound Authorisation Enforcement

Property	Value	Meaning
Reference Id	`HCM-FT-operation-bound-authorisation-enforcement`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-STEP2-OPERATION-BOUND-AUTHORISATION-ENFORCEMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Health Information Control	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33	Source standing does not approve the derived assertion.

31.80.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: general platform security/evidence Feature has no adjudicated patient-specific CT3 frame; established Health Information Control responsibility is preserved.

31.80.2. Feature Overview

Consistent enforcement is required across mutation, access, execution and administration rather than relying on each caller to voluntarily invoke a check.

31.80.3. Definition — Proposed

Apply effective policy at the governed action boundary and reject actions lacking sufficient authority.

31.80.4. Boundary

Consumes trusted identity and criteria; it does not authenticate or establish clinical privilege from directory roles.

31.80.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Health Information Control, rather than inheriting an adjacent responsibility.

31.80.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33

31.81. Feature: Entity-Relative Activity Coordination

Property	Value	Meaning
Reference Id	`HCM-FT-entity-relative-activity-coordination`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.FEAT-STEP2-ENTITY-RELATIVE-ACTIVITY-COORDINATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Entity-Specific Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.81.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal entity-relative progression is not the reviewed external work bridge; a current BEC destination has not been adjudicated.

31.81.2. Feature Overview

Interest matching can select multiple behaviours while the entity progresses one input at a time. Each behaviour uses a fresh managed information context.

31.81.3. Definition — Proposed

Sequence applicable behaviours for an identifiable entity input while preserving its operational reference frame.

31.81.4. Boundary

Fan-out is not concurrent execution; clinical decisions and originating information remain with their owners.

31.81.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Entity-Specific Operational Coordination, rather than inheriting an adjacent responsibility.

31.81.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.82. Feature: Entity Behaviour Failure & Recovery Governance

Property	Value	Meaning
Reference Id	`HCM-FT-entity-behaviour-failure-recovery-governance`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.FEAT-STEP2-ENTITY-BEHAVIOUR-FAILURE-RECOVERY-GOVERNANCE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Entity-Specific Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.82.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal entity-relative progression is not the reviewed external work bridge; a current BEC destination has not been adjudicated.

31.82.2. Feature Overview

Failure may stop a behaviour, an input or the entity progression and may leave effects uncertain. Resumption must preserve that distinction rather than imply rollback or automatic safe retry.

31.82.3. Definition — Proposed

Apply explicit behaviour-failure dispositions and controlled, authorised resumption for entity-relative processing.

31.82.4. Boundary

Recovery cannot silently declare no effects, bypass governance or resume a halted entity automatically.

31.82.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Entity-Specific Operational Coordination, rather than inheriting an adjacent responsibility.

31.82.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.83. Feature: Operational Activity Context Definition

Property	Value	Meaning
Reference Id	`HCM-FT-operational-activity-context-definition`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN.FEAT-STEP2-OPERATIONAL-ACTIVITY-CONTEXT-DEFINITION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Governed Operational Activity Design	Linked responsibility references; historical records do not assert current ancestry.
Evidence	context-and-persistence-management.md:52; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.83.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal execution-envelope design is distinct from governance of work across an external execution boundary.

31.83.2. Feature Overview

Even a one-unit activity needs a declared information and outcome frame. The definition is separate from loading values or running the business logic.

31.83.3. Definition — Proposed

Define the anticipated information context and expected fulfilment basis of an operational undertaking.

31.83.4. Boundary

No dynamically invented resource set or orchestration engine is implied.

31.83.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Governed Operational Activity Design, rather than inheriting an adjacent responsibility.

31.83.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task context-and-persistence-management.md:52; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.84. Feature: Activity Outcome & Continuation Rules

Property	Value	Meaning
Reference Id	`HCM-FT-activity-outcome-continuation-rules`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN.FEAT-STEP2-ACTIVITY-OUTCOME-CONTINUATION-RULES`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Governed Operational Activity Design	Linked responsibility references; historical records do not assert current ancestry.
Evidence	task-work.md:7; context-and-persistence-management.md:52	Source standing does not approve the derived assertion.

31.84.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal execution-envelope design is distinct from governance of work across an external execution boundary.

31.84.2. Feature Overview

An output may contribute to completion, subsequent activity or retained evidence under different rules. Those meanings need to be specified rather than inferred from a returned payload.

31.84.3. Definition — Proposed

Define how output roles, expected outcomes and continuation conditions are interpreted for an undertaking.

31.84.4. Boundary

Completion policy is not business outcome truth; uncertain effects remain explicit.

31.84.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Governed Operational Activity Design, rather than inheriting an adjacent responsibility.

31.84.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task task-work.md:7; context-and-persistence-management.md:52

31.85. Feature: Uncertain Activity Outcome Reconciliation

Property	Value	Meaning
Reference Id	`HCM-FT-uncertain-activity-outcome-reconciliation`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-STEP2-UNCERTAIN-ACTIVITY-OUTCOME-RECONCILIATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	foundational-requirements.md:76; 05-intrinsic-enablement.md:170	Source standing does not approve the derived assertion.

31.85.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.85.2. Feature Overview

A timeout or absent response leaves a materially significant outcome unknown. Reconciliation can confirm an effect, establish absence or retain uncertainty without automatically retrying the original work.

31.85.3. Definition — Proposed

Reconcile indeterminate activity effects through authoritative observation before establishing a safe next disposition.

31.85.4. Boundary

Execution uncertainty differs from insufficient assurance evidence; a retry does not prove the earlier attempt had no effect.

31.85.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Workflow & Activity Coordination, rather than inheriting an adjacent responsibility.

31.85.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task foundational-requirements.md:76; 05-intrinsic-enablement.md:170

31.86. Feature: Controlled Administrative Recovery

Property	Value	Meaning
Reference Id	`HCM-FT-controlled-administrative-recovery`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-STEP2-CONTROLLED-ADMINISTRATIVE-RECOVERY`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73; DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33	Source standing does not approve the derived assertion.

31.86.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.86.2. Feature Overview

Operators may need to resume, reconcile or repair an operational concern. That is first-class work with permission and outcome standing, rather than direct mutation through a console.

31.86.3. Definition — Proposed

Perform authorised recovery as identifiable governed activity through normal execution and evidence boundaries.

31.86.4. Boundary

Administrative interaction grants no raw cache/database or subsystem-internal bypass.

31.86.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Workflow & Activity Coordination, rather than inheriting an adjacent responsibility.

31.86.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73; DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33

31.87. Feature: Service Readiness Dependency Assessment

Property	Value	Meaning
Reference Id	`HCM-FT-service-readiness-dependency-assessment`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.

Property	Value	Meaning
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION.FEAT-STEP2-SERVICE-READINESS-DEPENDENCY-ASSESSMENT`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Care Resource & Capacity Coordination → BEC CT3: Service Offering Readiness Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	business-capabilities.md:403; information-responsibility.md:34	Source standing does not approve the derived assertion.

31.87.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: source proposal determines collective readiness; reviewed scope establishes dependency tracking only. Evidence may be reused, but determination authority is not silently transferred.

31.87.2. Feature Overview

An offering may be in operating hours but not deliverable because staffing, equipment, materials or supporting services are missing. A readiness view retains insufficiency and the owners of those dependency assertions.

31.87.3. Definition — Proposed

Associate time- and context-qualified dependency evidence to establish a bounded readiness view for an offering.

31.87.4. Boundary

Readiness evidence does not originate clinical clearance or transfer resource management.

31.87.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Service Offering Readiness Coordination, rather than inheriting an adjacent responsibility.

31.87.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

31.87.7. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:403; information-responsibility.md:34

31.88. Feature: Governance Requirement Version Publication

Property	Value	Meaning
Reference Id	`HCM-FT-governance-requirement-version-publication`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-GOVERNANCE-REQUIREMENT-PUBLICATION-SUPPORT.FEAT-STEP2-GOVERNANCE-REQUIREMENT-VERSION-PUBLICATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Protection & Accountability Enablement → BEC CT3: Governance Requirement Publication Support	Linked responsibility references; historical records do not assert current ancestry.
Evidence	business-capabilities.md:993; health-service-assurance.md:9	Source standing does not approve the derived assertion.

31.88.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Requirement publication/authority is not assurance-service review or information-model design; the reviewed hierarchy does not allocate a home.

31.88.2. Feature Overview

Operational and assurance consumers need to identify the governing basis they used rather than silently substitute a current rule for a historical one.

31.88.3. Definition — Proposed

Publish attributable versions and applicability of externally approved governing requirements.

31.88.4. Boundary

Publication does not confer requirement approval, authorisation policy or assurance-criteria authority.

31.88.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Governance Requirement Publication Support, rather than inheriting an adjacent responsibility.

31.88.6. Review Confidence

Low; applies to the proposal, not to preserved canonical meaning.

31.88.7. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task business-capabilities.md:993; health-service-assurance.md:9

31.89. Feature: Independent Evidence Assessment & Adjudication

Property	Value	Meaning
Reference Id	`HCM-FT-independent-evidence-assessment-adjudication`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	feature / FT	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FEAT-STEP2-INDEPENDENT-EVIDENCE-ASSESSMENT-ADJUDICATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Independent Assurance Enablement → BEC CT2: Independent Assurance Evaluation Enablement → BEC CT3: Governed Assurance	Linked responsibility references; historical records do not assert current ancestry.
Evidence	health-service-assurance.md:9	Source standing does not approve the derived assertion.

31.89.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical assurance against a governed subject is not independent review of the assurance service itself.

31.89.2. Feature Overview

The Feature unifies the review frame while retaining the three distinct canonical Functions and the boundary between criterion-level assessment and reportable conclusion.

31.89.3. Definition — Proposed

Independently establish assurance context, assess evidence and adjudicate findings under applicable criteria.

31.89.4. Boundary

No new assurance Function or clinical-adequacy mandate is introduced; insufficient evidence establishes neither satisfaction nor non-satisfaction.

31.89.5. Proposed Placement Rationale

This behaviour changes or exposes the bounded concern of Governed Assurance, rather than inheriting an adjacent responsibility.

31.89.6. Derivation Provenance

Inferred from capability-specific behaviour, v0.3 inspection and current architectural evidence. AI architectural synthesis under the explicitly authorised Step 2 task health-service-assurance.md:9

31.90. Function: Qualify Information Authority Standing

Property	Value	Meaning
Reference Id	`HCM-FN-qualify-information-authority-standing`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-INFORMATION-AUTHORITY-REPRESENTATION.FN-QUALIFY-INFORMATION-AUTHORITY-STANDING`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Clinical Information Stewardship Enablement → BEC CT3: Information Authority Representation	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:303	Source standing does not approve the derived assertion.

31.90.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Source credibility and authority standing require an explicit home; information-asset tags alone do not establish that responsibility.

31.90.2. Function Overview

Moves an unqualified received assertion to explicitly attributed authority standing without equating persistence success to credibility.

31.90.3. Definition — Proposed

Associate source and credibility qualifications with an information assertion under applicable governed semantics.

31.90.4. Boundary

Does not adjudicate all competing claims or originate clinical truth.

31.90.5. Proposed Placement Rationale

Behaviour remains within Information Authority Representation responsibility.

31.90.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:303

31.91. Function: Capture Required Significant Audit Evidence

Property	Value	Meaning
Reference Id	`HCM-FN-capture-required-significant-audit-evidence`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE.FN-CAPTURE-REQUIRED-SIGNIFICANT-AUDIT-EVIDENCE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Protection & Accountability Enablement → BEC CT3: Significant Activity Audit Capture	Linked responsibility references; historical records do not assert current ancestry.
Evidence	architectural-axioms.md:379	Source standing does not approve the derived assertion.

31.91.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The reviewed hierarchy does not allocate general mandatory authoritative audit capture; disclosure or provenance alone is not an equivalent responsibility.

31.91.2. Function Overview

Moves an auditable fact to confirmed captured standing or an explicit capture failure affecting the protected operation.

31.91.3. Definition — Proposed

Establish required significant activity evidence under the governed authoritative audit-capture responsibility.

31.91.4. Boundary

Specific capture authority is not borrowed from the originating identity and is not generalised.

31.91.5. Proposed Placement Rationale

Behaviour remains within Significant Activity Audit Capture responsibility.

31.91.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:379

31.92. Service: Mandatory Audit Evidence Capture

Property	Value	Meaning
Reference Id	`HCM-SV-mandatory-audit-evidence-capture`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE.SV-MANDATORY-AUDIT-EVIDENCE-CAPTURE`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Shared Information & Coordination Enablement → BEC CT2: Information Protection & Accountability Enablement → BEC CT3: Significant Activity Audit Capture	Linked responsibility references; historical records do not assert current ancestry.

Property	Value	Meaning
Evidence	architectural-axioms.md:379	Source standing does not approve the derived assertion.

31.92.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: The reviewed hierarchy does not allocate general mandatory authoritative audit capture; disclosure or provenance alone is not an equivalent responsibility.

31.92.2. Service Overview

The client contract distinguishes confirmed capture from inability to capture, allowing required operations to fail closed.

31.92.3. Definition — Proposed

Allow governed-operation boundaries to require authoritative capture of significant activity evidence.

31.92.4. Boundary

The exposed boundary does not permit callers to assign themselves audit platform authority.

31.92.5. Proposed Placement Rationale

Behaviour remains within Significant Activity Audit Capture responsibility.

31.92.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task architectural-axioms.md:379

31.93. Function: Coordinate Entity-Relative Input

Property	Value	Meaning
Reference Id	`HCM-FN-coordinate-entity-relative-input`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.FN-COORDINATE-ENTITY-RELATIVE-INPUT`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Entity-Specific Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.93.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal entity-relative progression is not the reviewed external work bridge; a current BEC destination has not been adjudicated.

31.93.2. Function Overview

Moves an accepted input through the applicable stable behaviour sequence using fresh managed contexts.

31.93.3. Definition — Proposed

Select interested behaviours and progress one entity-relative input with explicit participation and effect standing.

31.93.4. Boundary

No concurrent intra-entity progression or implicit clinical authority is introduced.

31.93.5. Proposed Placement Rationale

Behaviour remains within Entity-Specific Operational Coordination responsibility.

31.93.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.94. Service: Entity Operational Input Submission

Property	Value	Meaning
Reference Id	`HCM-SV-entity-operational-input-submission`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.

Property	Value	Meaning
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.SV-ENTITY-OPERATIONAL-INPUT-SUBMISSION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Entity-Specific Operational Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.94.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal entity-relative progression is not the reviewed external work bridge; a current BEC destination has not been adjudicated.

31.94.2. Service Overview

Acceptance concerns the stated handoff contract; the caller observes completion/effect through separately qualified progression information.

31.94.3. Definition — Proposed

Allow authorised domain coordinators to submit an entity-relative stimulus or requested operational effect.

31.94.4. Boundary

Input acceptance does not establish successful completion or authority over source clinical facts.

31.94.5. Proposed Placement Rationale

Behaviour remains within Entity-Specific Operational Coordination responsibility.

31.94.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.95. Function: Maintain Operational Fulfilment Definition

Property	Value	Meaning
Reference Id	`HCM-FN-maintain-operational-fulfilment-definition`	Stable concept identity.

Property	Value	Meaning
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN.FN-MAINTAIN-OPERATIONAL-FULFILMENT-DEFINITION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Governed Operational Activity Design	Linked responsibility references; historical records do not assert current ancestry.
Evidence	context-and-persistence-management.md:52; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.95.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Internal execution-envelope design is distinct from governance of work across an external execution boundary.

31.95.2. Function Overview

Changes governed activity design while preserving the applicability of the definition used by an existing undertaking.

31.95.3. Definition — Proposed

Establish and revise the declared context needs, expected outcome basis and housekeeping of an operational undertaking.

31.95.4. Boundary

A definition revision is not a command to run work or a dynamic clinical workflow generator.

31.95.5. Proposed Placement Rationale

Behaviour remains within Governed Operational Activity Design responsibility.

31.95.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task context-and-persistence-management.md:52; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.96. Function: Reconcile Indeterminate Activity Effects

Property	Value	Meaning
Reference Id	`HCM-FN-reconcile-indeterminate-activity-effects`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-RECONCILE-INDETERMINATE-ACTIVITY-EFFECTS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	05-intrinsic-enablement.md:170; foundational-requirements.md:76	Source standing does not approve the derived assertion.

31.96.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.96.2. Function Overview

Moves an uncertainty concern to a justified disposition while preserving the possibility that the original attempt produced an effect.

31.96.3. Definition — Proposed

Inspect authoritative observations to establish, qualify or retain uncertainty about a previously indeterminate activity effect.

31.96.4. Boundary

Missing evidence does not establish absence of effect and cannot automatically justify a repeat mutation.

31.96.5. Proposed Placement Rationale

Behaviour remains within Workflow & Activity Coordination responsibility.

31.96.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task 05-intrinsic-enablement.md:170; foundational-requirements.md:76

31.97. Function: Establish Activity Effect Observation Basis

Property	Value	Meaning
Reference Id	`HCM-FN-establish-activity-effect-observation-basis`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-ESTABLISH-ACTIVITY-EFFECT-OBSERVATION-BASIS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	05-intrinsic-enablement.md:170; context-and-persistence-management.md:52	Source standing does not approve the derived assertion.

31.97.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.97.2. Function Overview

Changes an unqualified uncertainty concern into a time- and attempt-specific observation basis for reconciliation.

31.97.3. Definition — Proposed

Associate the applicable handoff contract and authoritative observations with an indeterminate operational undertaking.

31.97.4. Boundary

Source observations retain their authority; this is an operational observation basis rather than an independent assurance assessment.

31.97.5. Proposed Placement Rationale

Behaviour remains within Workflow & Activity Coordination responsibility.

31.97.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task 05-intrinsic-enablement.md:170; context-and-persistence-management.md:52

31.98. Function: Determine Reconciled Activity Continuation

Property	Value	Meaning
Reference Id	`HCM-FN-determine-reconciled-activity-continuation`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-DETERMINE-RECONCILED-ACTIVITY-CONTINUATION`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	05-intrinsic-enablement.md:170; DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33	Source standing does not approve the derived assertion.

31.98.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.98.2. Function Overview

Moves a reconciled undertaking to justified continuation, authorised retry or explicitly retained uncertainty.

31.98.3. Definition — Proposed

Establish the permitted operational continuation from reconciled effect standing and applicable authorisation.

31.98.4. Boundary

Does not confer clinical decision authority or infer absence of effects from silence; policy evaluation remains separately owned.

31.98.5. Proposed Placement Rationale

Behaviour remains within Workflow & Activity Coordination responsibility.

31.98.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task 05-intrinsic-enablement.md:170; DesignNote-Identity-Authentication-and-Authorisation-Architecture.md:33

31.99. Process: Indeterminate Activity Outcome Reconciliation Process

Property	Value	Meaning
Reference Id	`HCM-PR-indeterminate-activity-outcome-reconciliation-process`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	process / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-INDETERMINATE-ACTIVITY-OUTCOME-RECONCILIATION-PROCESS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Human Collaboration & Work Enablement → BEC CT2: Operational Activity & Temporal Enablement → BEC CT3: Workflow & Activity Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	05-intrinsic-enablement.md:170; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73	Source standing does not approve the derived assertion.

31.99.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: Canonical internal Work Order, To Do and synthetic Task progression is broader than the reviewed external execution-boundary bridge.

31.99.2. Process Overview

First inspect the governing handoff and available authoritative state; then reconcile observed effects; finally authorise a safe continuation, retry or retained uncertainty. Each participating Function keeps its responsibility owner.

31.99.3. Definition — Proposed

Order the observation, effect qualification and authorised disposition of a materially indeterminate operational outcome.

31.99.4. Boundary

Not a universal lifecycle or automatic retry loop. A safe disposition may remain unresolved.

31.99.5. Proposed Placement Rationale

Behaviour remains within Workflow & Activity Coordination responsibility.

31.99.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task 05-intrinsic-enablement.md:170; DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md:73

31.99.7. Proposed Ordered Function Chain

Function: Establish Activity Effect Observation Basis (p. 991) → Function: Reconcile Indeterminate Activity Effects (p. 990) → Function: Determine Reconciled Activity Continuation (p. 992)

31.100. Function: Assess Service Offering Readiness

Property	Value	Meaning
Reference Id	`HCM-FN-assess-service-offering-readiness`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	function / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION.FN-ASSESS-SERVICE-OFFERING-READINESS`	Reserved historical allocation; does not assert current placement.
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Care Resource & Capacity Coordination → BEC CT3: Service Offering Readiness Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	information-responsibility.md:34; business-capabilities.md:403	Source standing does not approve the derived assertion.

31.100.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: source proposal determines collective readiness; reviewed scope establishes dependency tracking only. Evidence may be reused, but determination authority is not silently transferred.

31.100.2. Function Overview

Combines qualified dependency standing without converting missing evidence into ready or assuming that an available resource establishes deliverability.

31.100.3. Definition — Proposed

Evaluate applicable dependency evidence to establish a context- and time-qualified service readiness view.

31.100.4. Boundary

Resource ownership and clinical clearance remain external to this coordination.

31.100.5. Proposed Placement Rationale

Behaviour remains within Service Offering Readiness Coordination responsibility.

31.100.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34; business-capabilities.md:403

31.101. Service: Service Offering Readiness Query

Property	Value	Meaning
Reference Id	`HCM-SV-service-offering-readiness-query`	Stable concept identity.
Reconciliation standing	pending adjudication	Preserved outside the current hierarchy; original semantic standing is unchanged.
Element status	Proposed	Step 2 architectural proposal.
Type / tier	service / behaviour	Type is independent of heading depth.
Direct placement	Proposed	Immediate context; complete path status is separate.
Complete structural assertion	Proposed	Proposed ancestry does not demote a Canonical responsibility.
Last structural ID — historical	`CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION.SV-SERVICE-OFFERING-READINESS-QUERY`	Reserved historical allocation; does not assert current placement.

Property	Value	Meaning
Preserved context references	BEC CT1: Health Service Operations Enablement → BEC CT2: Care Resource & Capacity Coordination → BEC CT3: Service Offering Readiness Coordination	Linked responsibility references; historical records do not assert current ancestry.
Evidence	information-responsibility.md:34	Source standing does not approve the derived assertion.

31.101.1. HUMAN ADJUDICATION REQUIRED

HUMAN ADJUDICATION REQUIRED: source proposal determines collective readiness; reviewed scope establishes dependency tracking only. Evidence may be reused, but determination authority is not silently transferred.

31.101.2. Service Overview

The response identifies relevant evidence basis and insufficiency rather than an unconditional ready flag.

31.101.3. Definition — Proposed

Expose the qualified readiness view of a defined offering and operating context to operational planning consumers.

31.101.4. Boundary

A readiness answer grants no resource allocation or clinical go/no-go authority.

31.101.5. Proposed Placement Rationale

Behaviour remains within Service Offering Readiness Coordination responsibility.

31.101.6. Derivation Provenance

Strongly implied bounded information/work responsibility. AI architectural synthesis under the explicitly authorised Step 2 task information-responsibility.md:34

32. Relationship register

An edge has its own assertion status. Canonical endpoints do not promote a Proposed edge. Unresolved associations are retained non-edges. Enablement does not mean decomposition, runtime dependency, implementation equivalence or transfer of originating authority; mappings are not inherited by descendants.

32.1. Capability Decomposition

From	To	Assertion / rationale
BC CT1: Individual Care Delivery (p. 5)	BC CT2: Individual Health Assessment (p. 6)	Established business:162 (p. 1333)
BC CT1: Individual Care Delivery (p. 5)	BC CT2: Clinical Determination (p. 10)	Established business:188 (p. 1333)
BC CT1: Individual Care Delivery (p. 5)	BC CT2: Individual Care Planning (p. 14)	Established business:213 (p. 1334)
BC CT1: Individual Care Delivery (p. 5)	BC CT2: Clinical Intervention (p. 18)	Established business:239 (p. 1334)
BC CT1: Individual Care Delivery (p. 5)	BC CT2: Individual Health Monitoring (p. 19)	Established business:265 (p. 1334)
BC CT1: Individual Care Delivery (p. 5)	BC CT2: Individual Care Review (p. 22)	Established business:291 (p. 1335)
BC CT2: Individual Health Assessment (p. 6)	BC CT3: Health History Assessment (p. 7)	Established business:326 (p. 1335); business:322 (p. 1335)
BC CT2: Individual Health Assessment (p. 6)	BC CT3: Clinical Examination (p. 8)	Established business:327 (p. 1335); business:322 (p. 1335)
BC CT2: Individual Health Assessment (p. 6)	BC CT3: Health Status Measurement (p. 9)	Established business:328 (p. 1335); business:322 (p. 1335)
BC CT2: Individual Health Monitoring (p. 19)	BC CT3: Health State Monitoring (p. 20)	Established business:342 (p. 1336); business:338 (p. 1336)
BC CT2: Individual Health Monitoring (p. 19)	BC CT3: Care Response Monitoring (p. 21)	Established business:343 (p. 1336); business:338 (p. 1336)
BC CT2: Individual Care Review (p. 22)	BC CT3: Care Appropriateness Review (p. 23)	Established business:357 (p. 1336); business:353 (p. 1336)
BC CT2: Individual Care Review (p. 22)	BC CT3: Care Effectiveness Review (p. 24)	Established business:358 (p. 1336); business:353 (p. 1336)
BC CT1: Care Access & Coordination (p. 26)	BC CT2: Care Access Management (p. 27)	Established business:372 (p. 1336)
BC CT2: Care Access Management (p. 27)	BC CT3: Care Intake Management (p. 28)	Established business:380 (p. 1336); business:376 (p. 1336)
BC CT2: Care Access Management (p. 27)	BC CT3: Care Referral Management (p. 29)	Established business:381 (p. 1336); business:376 (p. 1336)
BC CT2: Care Access Management (p. 27)	BC CT3: Care Eligibility Determination (p. 30)	Established business:382 (p. 1336); business:376 (p. 1336)

From	To	Assertion / rationale
BC CT1: Care Access & Coordination (p. 26)	BC CT2: Care Scheduling & Capacity Management (p. 31)	Established business:388 (p. 1337)
BC CT2: Care Scheduling & Capacity Management (p. 31)	BC CT3: Care Appointment Management (p. 32)	Established business:396 (p. 1337); business:392 (p. 1337)
BC CT2: Care Scheduling & Capacity Management (p. 31)	BC CT3: Care-Place Allocation (p. 33)	Established business:397 (p. 1337); business:392 (p. 1337)
BC CT2: Care Scheduling & Capacity Management (p. 31)	BC CT3: Care-Place Readiness Management (p. 34)	Established business:398 (p. 1337); business:392 (p. 1337)
BC CT2: Care Scheduling & Capacity Management (p. 31)	BC CT3: Care Resource Deployment (p. 34)	Established business:399 (p. 1337); business:392 (p. 1337)
BC CT2: Care Scheduling & Capacity Management (p. 31)	BC CT3: Operational Care Flow Coordination (p. 35)	Established business:400 (p. 1337); business:392 (p. 1337)
BC CT2: Care Scheduling & Capacity Management (p. 31)	BC CT3: Operational Care Capacity Management (p. 36)	Established business:401 (p. 1337); business:392 (p. 1337)
BC CT1: Care Access & Coordination (p. 26)	BC CT2: Care Coordination & Transition (p. 37)	Established business:407 (p. 1337)
BC CT2: Care Coordination & Transition (p. 37)	BC CT3: Care-Team Coordination (p. 38)	Established business:415 (p. 1337); business:411 (p. 1337)
BC CT2: Care Coordination & Transition (p. 37)	BC CT3: Care Responsibility Transition (p. 39)	Established business:416 (p. 1337); business:411 (p. 1337)
BC CT2: Care Coordination & Transition (p. 37)	BC CT3: Care Discharge Coordination (p. 40)	Established business:417 (p. 1337); business:411 (p. 1337)
BC CT1: Care Access & Coordination (p. 26)	BC CT2: Care Support Logistics Management (p. 41)	Established business:423 (p. 1338)
BC CT2: Care Support Logistics Management (p. 41)	BC CT3: Patient Movement Coordination (p. 42)	Established business:431 (p. 1338); business:427 (p. 1338)
BC CT2: Care Support Logistics Management (p. 41)	BC CT3: Clinical Material Logistics Coordination (p. 43)	Established business:432 (p. 1338); business:427 (p. 1338)
BC CT2: Care Support Logistics Management (p. 41)	BC CT3: Care Support Work Coordination (p. 44)	Established business:433 (p. 1338); business:427 (p. 1338)
BC CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 50)	BC CT2: Diagnostic Support Provision (p. 51)	Established business:459 (p. 1338)
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Service Appointment Management (p. 52)	Established business:467 (p. 1339); business:463 (p. 1339)
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Service Delivery (p. 53)	Established business:468 (p. 1339); business:463 (p. 1339)
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Specimen Management (p. 54)	Established business:469 (p. 1339); business:463 (p. 1339)
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Data Management (p. 55)	Established business:470 (p. 1339); business:463 (p. 1339)

From	To	Assertion / rationale
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Interpretation (p. 56)	Established business:471 (p. 1339); business:463 (p. 1339)
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Report Management (p. 56)	Established business:472 (p. 1339); business:463 (p. 1339)
BC CT2: Diagnostic Support Provision (p. 51)	BC CT3: Diagnostic Report Distribution (p. 57)	Established business:473 (p. 1339); business:463 (p. 1339)
BC CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 50)	BC CT2: Medication & Pharmacy Services (p. 58)	Established business:479 (p. 1339)
BC CT2: Medication & Pharmacy Services (p. 58)	BC CT3: Prescription Medication Dispensing (p. 60)	Established business:487 (p. 1339); business:483 (p. 1339)
BC CT2: Medication & Pharmacy Services (p. 58)	BC CT3: Non-Prescription Medication Provision (p. 60)	Established business:488 (p. 1339); business:483 (p. 1339)
BC CT2: Medication & Pharmacy Services (p. 58)	BC CT3: Medication Advice (p. 61)	Established business:489 (p. 1339); business:483 (p. 1339)
BC CT2: Medication & Pharmacy Services (p. 58)	BC CT3: Medication Dose Administration Aid Provision (p. 62)	Established business:490 (p. 1339); business:483 (p. 1339)
BC CT1: Health Products & Clinical Technology (p. 65)	BC CT2: Clinical Technology Management (p. 66)	Established business:504 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Inventory Management (p. 67)	Established business:512 (p. 1340); business:508 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Feature & Release Management (p. 68)	Established business:513 (p. 1340); business:508 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Loan & Deployment Management (p. 69)	Established business:514 (p. 1340); business:508 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Utilisation Support (p. 70)	Established business:515 (p. 1340); business:508 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Integration Provision (p. 71)	Established business:516 (p. 1340); business:508 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Data Integration (p. 71)	Established business:517 (p. 1340); business:508 (p. 1340)
BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Lifecycle Management (p. 72)	Established business:518 (p. 1340); business:508 (p. 1340)
BC CT1: Clinical Quality, Safety & Improvement (p. 75)	BC CT2: Clinical Safety Management (p. 76)	Established business:532 (p. 1341)
BC CT2: Clinical Safety Management (p. 76)	BC CT3: Clinical Risk Identification (p. 77)	Established business:540 (p. 1341); business:536 (p. 1341)
BC CT2: Clinical Safety Management (p. 76)	BC CT3: Clinical Risk Mitigation Management (p. 78)	Established business:541 (p. 1341); business:536 (p. 1341)
BC CT2: Clinical Safety Management (p. 76)	BC CT3: Clinical Risk Response Management (p. 79)	Established business:542 (p. 1341); business:536 (p. 1341)

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BC CT1: Clinical Quality, Safety & Improvement (p. 75)	BC CT2: Clinical Quality Management (p. 79)	Established business:548 (p. 1341)
BC CT2: Clinical Quality Management (p. 79)	BC CT3: Clinical Quality Criteria Management (p. 81)	Established business:556 (p. 1341); business:552 (p. 1341)
BC CT2: Clinical Quality Management (p. 79)	BC CT3: Clinical Quality Evidence Management (p. 81)	Established business:557 (p. 1341); business:552 (p. 1341)
BC CT2: Clinical Quality Management (p. 79)	BC CT3: Clinical Quality Review & Assessment Management (p. 82)	Established business:558 (p. 1341); business:552 (p. 1341)
BC CT2: Clinical Quality Management (p. 79)	BC CT3: Clinical Quality Reporting (p. 83)	Established business:559 (p. 1341); business:552 (p. 1341)
BC CT1: Community Health & Wellbeing (p. 85)	BC CT2: Preventative Health (p. 86)	Established business:573 (p. 1342)
BC CT2: Preventative Health (p. 86)	BC CT3: Preventative Health Problem Identification (p. 87)	Established business:581 (p. 1342); business:577 (p. 1342)
BC CT2: Preventative Health (p. 86)	BC CT3: Preventative Health Programme Management (p. 88)	Established business:582 (p. 1342); business:577 (p. 1342)
BC CT2: Preventative Health (p. 86)	BC CT3: Preventative Health Education Services (p. 89)	Established business:583 (p. 1342); business:577 (p. 1342)
BC CT2: Preventative Health (p. 86)	BC CT3: Preventative Health Programme Monitoring (p. 90)	Established business:584 (p. 1342); business:577 (p. 1342)
BC CT2: Preventative Health (p. 86)	BC CT3: Preventative Health Reporting (p. 90)	Established business:585 (p. 1342); business:577 (p. 1342)
BC CT2: Preventative Health (p. 86)	BC CT3: Preventative Health Cohort Management (p. 91)	Established business:586 (p. 1342); business:577 (p. 1342)
BC CT1: Population Health & Health-System Planning (p. 93)	BC CT2: Population Health Surveillance (p. 94)	Established business:600 (p. 1342)
BC CT2: Population Health Surveillance (p. 94)	BC CT3: Population Health Longitudinal Assessment (p. 95)	Established business:608 (p. 1342); business:604 (p. 1342)
BC CT2: Population Health Surveillance (p. 94)	BC CT3: Population Health Surveillance Management (p. 96)	Established business:609 (p. 1342); business:604 (p. 1342)
BC CT2: Population Health Surveillance (p. 94)	BC CT3: Population Health Event Identification (p. 97)	Established business:610 (p. 1342); business:604 (p. 1342)
BC CT2: Population Health Surveillance (p. 94)	BC CT3: Notifiable Health Event Management (p. 98)	Established business:611 (p. 1342); business:604 (p. 1342)
BC CT2: Population Health Surveillance (p. 94)	BC CT3: Population Health Reporting (p. 99)	Established business:612 (p. 1342); business:604 (p. 1342)
BC CT1: Population Health & Health-System Planning (p. 93)	BC CT2: Health-System Planning (p. 99)	Established business:618 (p. 1343)
BC CT2: Health-System Planning (p. 99)	BC CT3: Health Service Demand Assessment (p. 101)	Established business:626 (p. 1343); business:622 (p. 1343)

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BC CT2: Health-System Planning (p. 99)	BC CT3: Health Service Capacity Planning (p. 102)	Established business:627 (p. 1343); business:622 (p. 1343)
BC CT2: Health-System Planning (p. 99)	BC CT3: Health Service Distribution Planning (p. 103)	Established business:628 (p. 1343); business:622 (p. 1343)
BC CT1: Health Information & Knowledge Management (p. 104)	BC CT2: Health Information Management (p. 105)	Established business:646 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Data Management (p. 106)	Established business:654 (p. 1344); business:650 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Structure Management (p. 107)	Established business:655 (p. 1344); business:650 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Lifecycle Management (p. 108)	Established business:656 (p. 1344); business:650 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Catalogue and Index Management (p. 109)	Established business:657 (p. 1344); business:650 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Compartmentalisation (p. 110)	Established business:658 (p. 1344); business:650 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Data Correlation (p. 111)	Established business:659 (p. 1344); business:650 (p. 1343)
BC CT2: Health Information Management (p. 105)	BC CT3: Health Information Data Retrieval (p. 112)	Established business:660 (p. 1344); business:650 (p. 1343)
BC CT1: Health Information & Knowledge Management (p. 104)	BC CT2: Clinical Knowledge Management (p. 112)	Established business:666 (p. 1344)
BC CT2: Clinical Knowledge Management (p. 112)	BC CT3: Clinical Knowledge Context Identification (p. 114)	Established business:674 (p. 1344); business:670 (p. 1344)
BC CT2: Clinical Knowledge Management (p. 112)	BC CT3: Clinical Knowledge Context Extraction (p. 115)	Established business:675 (p. 1344); business:670 (p. 1344)
BC CT2: Clinical Knowledge Management (p. 112)	BC CT3: Clinical Knowledge Information Formulation (p. 116)	Established business:676 (p. 1344); business:670 (p. 1344)
BC CT2: Clinical Knowledge Management (p. 112)	BC CT3: Clinical Knowledge Concept Association (p. 116)	Established business:677 (p. 1344); business:670 (p. 1344)
BC CT2: Clinical Knowledge Management (p. 112)	BC CT3: Clinical Knowledge Review & Approval Management (p. 117)	Established business:678 (p. 1344); business:670 (p. 1344)
BC CT2: Clinical Knowledge Management (p. 112)	BC CT3: Clinical Knowledge Publication & Sharing (p. 118)	Established business:679 (p. 1344); business:670 (p. 1344)
BC CT1: Standards, Semantics & Reference Governance (p. 120)	BC CT2: Health Information Standards Governance (p. 121)	Established business:693 (p. 1345)
BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Information Model Discovery (p. 122)	Established business:701 (p. 1345); business:697 (p. 1345)

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BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Information Model Adoption (p. 123)	Established business:702 (p. 1345); business:697 (p. 1345)
BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Information Model Education (p. 124)	Established business:703 (p. 1345); business:697 (p. 1345)
BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Information Model Compliance (p. 125)	Established business:704 (p. 1345); business:697 (p. 1345)
BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Information Model Governance Oversight (p. 125)	Established business:705 (p. 1345); business:697 (p. 1345)
BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Standards Adoption (p. 126)	Established business:706 (p. 1345); business:697 (p. 1345)
BC CT2: Health Information Standards Governance (p. 121)	BC CT3: Standards Impact Identification (p. 127)	Established business:707 (p. 1345); business:697 (p. 1345)
BC CT1: Standards, Semantics & Reference Governance (p. 120)	BC CT2: Clinical Semantics Governance (p. 128)	Established business:713 (p. 1345)
BC CT2: Clinical Semantics Governance (p. 128)	BC CT3: Terminology Governance (p. 129)	Established business:721 (p. 1345); business:717 (p. 1345)
BC CT2: Clinical Semantics Governance (p. 128)	BC CT3: Terminology Utilisation Oversight (p. 130)	Established business:722 (p. 1345); business:717 (p. 1345)
BC CT2: Clinical Semantics Governance (p. 128)	BC CT3: Terminology Localisation (p. 131)	Established business:723 (p. 1345); business:717 (p. 1345)
BC CT2: Clinical Semantics Governance (p. 128)	BC CT3: Classification Governance (p. 132)	Established business:724 (p. 1345); business:717 (p. 1345)
BC CT2: Clinical Semantics Governance (p. 128)	BC CT3: Classification Utilisation Oversight (p. 133)	Established business:725 (p. 1345); business:717 (p. 1345)
BC CT2: Clinical Semantics Governance (p. 128)	BC CT3: Semantic Mapping Governance (p. 133)	Established business:726 (p. 1345); business:717 (p. 1345)
BC CT1: Connected Health Services (p. 135)	BC CT2: Health Information Exchange (p. 136)	Established business:740 (p. 1346)
BC CT1: Connected Health Services (p. 135)	BC CT2: Digital Communication Channel (p. 138)	Established business:750 (p. 1346)
BC CT1: Security, Privacy & Digital Trust (p. 140)	BC CT2: Digital Identity & Access Governance (p. 141)	Established business:768 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Identity Assurance (p. 142)	Established business:776 (p. 1347); business:772 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Identity Modelling & Interoperability (p. 143)	Established business:777 (p. 1347); business:772 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Identity Qualification Control (p. 144)	Established business:778 (p. 1347); business:772 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Identity Identification & Authentication (p. 145)	Established business:779 (p. 1347); business:772 (p. 1347)

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BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Access Control Context Management (p. 146)	Established business:780 (p. 1347); business:772 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Access Control Assurance & Reporting (p. 147)	Established business:781 (p. 1347); business:772 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Access Control Collaboration (p. 147)	Established business:782 (p. 1347); business:772 (p. 1347)
BC CT2: Digital Identity & Access Governance (p. 141)	BC CT3: Access Control Enforcement (p. 148)	Established business:783 (p. 1347); business:772 (p. 1347)
BC CT1: Security, Privacy & Digital Trust (p. 140)	BC CT2: Health Information Protection (p. 149)	Established business:789 (p. 1347)
BC CT2: Health Information Protection (p. 149)	BC CT3: Health Privacy Safeguard Management (p. 150)	Established business:797 (p. 1347); business:793 (p. 1347)
BC CT2: Health Information Protection (p. 149)	BC CT3: Health Information Safeguarding (p. 151)	Established business:798 (p. 1347); business:793 (p. 1347)
BC CT2: Health Information Protection (p. 149)	BC CT3: Health Information Access Monitoring (p. 152)	Established business:799 (p. 1347); business:793 (p. 1347)
BC CT2: Health Information Protection (p. 149)	BC CT3: Health Information Compartment Assurance (p. 153)	Established business:800 (p. 1347); business:793 (p. 1347)
BC CT2: Health Information Protection (p. 149)	BC CT3: Health Information Classification Assurance (p. 154)	Established business:801 (p. 1347); business:793 (p. 1347)
BC CT1: Security, Privacy & Digital Trust (p. 140)	BC CT2: Digital Accountability & Trust (p. 155)	Established business:807 (p. 1348)
BC CT2: Digital Accountability & Trust (p. 155)	BC CT3: Governed Activity Attribution (p. 156)	Established business:815 (p. 1348); business:811 (p. 1348)
BC CT2: Digital Accountability & Trust (p. 155)	BC CT3: Security & Information-Use Accountability (p. 157)	Established business:816 (p. 1348); business:811 (p. 1348)
BC CT1: Health Research & Innovation (p. 159)	BC CT2: Research Information Support (p. 160)	Established business:834 (p. 1348)
BC CT2: Research Information Support (p. 160)	BC CT3: Governed Research Information Provision (p. 161)	Established business:842 (p. 1349); business:838 (p. 1348)
BC CT1: Health Research & Innovation (p. 159)	BC CT2: Health Innovation (p. 162)	Established business:848 (p. 1349)
BC CT2: Health Innovation (p. 162)	BC CT3: Health Innovation Evaluation (p. 163)	Established business:856 (p. 1349); business:852 (p. 1349)
BC CT1: Health Service Direction & Stewardship (p. 165)	BC CT2: Health Service Modelling (p. 166)	Established business:874 (p. 1349)
BC CT2: Health Service Modelling (p. 166)	BC CT3: Health Service Need Identification (p. 167)	Established business:882 (p. 1349); business:878 (p. 1349)
BC CT2: Health Service Modelling (p. 166)	BC CT3: Health Service Capability Identification (p. 168)	Established business:883 (p. 1349); business:878 (p. 1349)

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BC CT2: Health Service Modelling (p. 166)	BC CT3: Health Service Implementation Support (p. 169)	Established business:884 (p. 1349); business:878 (p. 1349)
BC CT2: Health Service Modelling (p. 166)	BC CT3: Health Service Continuous Improvement (p. 170)	Established business:885 (p. 1349); business:878 (p. 1349)
BC CT1: Health Service Direction & Stewardship (p. 165)	BC CT2: Health Service Alignment (p. 170)	Established business:891 (p. 1350)
BC CT2: Health Service Alignment (p. 170)	BC CT3: Health Service Co-Dependency Management (p. 172)	Established business:899 (p. 1350); business:895 (p. 1350)
BC CT2: Health Service Alignment (p. 170)	BC CT3: Health Service Resource Identification (p. 172)	Established business:900 (p. 1350); business:895 (p. 1350)
BC CT2: Health Service Alignment (p. 170)	BC CT3: Health Service Financial Management (p. 173)	Established business:901 (p. 1350); business:895 (p. 1350)
BC CT2: Health Service Alignment (p. 170)	BC CT3: Health Service Implementation Planning (p. 174)	Established business:902 (p. 1350); business:895 (p. 1350)
BC CT1: Workforce & Organisational Capability (p. 176)	BC CT2: Health Workforce Management (p. 177)	Established business:918 (p. 1350)
BC CT2: Health Workforce Management (p. 177)	BC CT3: Practitioner Qualification Management (p. 178)	Established business:926 (p. 1350); business:922 (p. 1350)
BC CT2: Health Workforce Management (p. 177)	BC CT3: Practitioner Engagement & Privilege Management (p. 179)	Established business:927 (p. 1350); business:922 (p. 1350)
BC CT2: Health Workforce Management (p. 177)	BC CT3: Workforce Service Coverage Management (p. 180)	Established business:928 (p. 1350); business:922 (p. 1350)
BC CT1: Workforce & Organisational Capability (p. 176)	BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	Established business:934 (p. 1351)
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	BC CT3: Healthcare Organisation Identity & Structure Management (p. 182)	Established business:942 (p. 1351); business:938 (p. 1351)
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	BC CT3: Healthcare Location Definition Management (p. 183)	Established business:943 (p. 1351); business:938 (p. 1351)
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	BC CT3: Healthcare Service Offering Management (p. 184)	Established business:944 (p. 1351); business:938 (p. 1351)
BC CT1: Corporate Resources & Enterprise Services (p. 185)	BC CT2: Financial & Commercial Services (p. 186)	Established business:958 (p. 1351)
BC CT2: Financial & Commercial Services (p. 186)	BC CT3: Clinical Activity Charging Information Provision (p. 187)	Established business:966 (p. 1352); business:962 (p. 1351)
BC CT1: Corporate Resources & Enterprise Services (p. 185)	BC CT2: Corporate Support Services (p. 188)	Established business:972 (p. 1352)

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BC CT2: Corporate Support Services (p. 188)	BC CT3: Back-Office Services (p. 189)	Established business:980 (p. 1352); business:976 (p. 1352)
BC CT2: Corporate Support Services (p. 188)	BC CT3: Facilities Management (p. 190)	Established business:981 (p. 1352); business:976 (p. 1352)
BC CT2: Corporate Support Services (p. 188)	BC CT3: IT Infrastructure Management (p. 191)	Established business:982 (p. 1352); business:976 (p. 1352)
BC CT2: Corporate Support Services (p. 188)	BC CT3: Communications & Publicity (p. 192)	Established business:983 (p. 1352); business:976 (p. 1352)
BC CT2: Corporate Support Services (p. 188)	BC CT3: People Management (p. 192)	Established business:984 (p. 1352); business:976 (p. 1352)
BC CT2: Corporate Support Services (p. 188)	BC CT3: Operational Oversight (p. 193)	Established business:985 (p. 1352); business:976 (p. 1352)
BC CT1: Health Service Governance (p. 195)	BC CT2: Governance Framework Management (p. 196)	Established business:1005 (p. 1353)
BC CT2: Governance Framework Management (p. 196)	BC CT3: Policy / Obligation Governance (p. 197)	Established business:1013 (p. 1353); business:1009 (p. 1353)
BC CT2: Governance Framework Management (p. 196)	BC CT3: Authority & Constraint Governance (p. 198)	Established business:1014 (p. 1353); business:1009 (p. 1353)
BC CT1: Health Service Governance (p. 195)	BC CT2: Compliance Governance (p. 199)	Established business:1020 (p. 1353)
BC CT2: Compliance Governance (p. 199)	BC CT3: Compliance Assessment Definition (p. 200)	Established business:1028 (p. 1353); business:1024 (p. 1353)
BC CT2: Compliance Governance (p. 199)	BC CT3: Governance Reporting & Checkpoint Definition (p. 201)	Established business:1029 (p. 1353); business:1024 (p. 1353)
BC CT1: Health Service Assurance (p. 202)	BC CT2: Health Service Assurance Specification (p. 203)	Established business:1045 (p. 1354)
BC CT2: Health Service Assurance Specification (p. 203)	BC CT3: Assurance Approach Definition (p. 204)	Established business:1053 (p. 1354); business:1049 (p. 1354)
BC CT2: Health Service Assurance Specification (p. 203)	BC CT3: Assurance Evaluation Criteria Governance (p. 205)	Established business:1054 (p. 1354); business:1049 (p. 1354)
BC CT1: Health Service Assurance (p. 202)	BC CT2: Assurance Evaluation (p. 206)	Established business:1060 (p. 1354)
BC CT2: Assurance Evaluation (p. 206)	BC CT3: Assurance Evidence Assessment (p. 207)	Established business:1068 (p. 1354); business:1064 (p. 1354)
BC CT2: Assurance Evaluation (p. 206)	BC CT3: Assurance Conclusion Determination (p. 208)	Established business:1069 (p. 1354); business:1064 (p. 1354)
BC CT2: Service Delivery Preparedness Management (p. 44)	BC CT3: Health Service Operational Readiness (p. 46)	Proposed Collective readiness is an access prerequisite and has a coherent proposed preparedness frame; it is not reduced to capacity, resource state or service alignment.

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BC CT2: Clinical Technology Management (p. 66)	BC CT3: Clinical Device Maintenance Management (p. 73)	Proposed Maintenance/restoration is materially distinct from overall lifecycle disposition and monitoring; include the Candidate concern as a proposed eighth CT3.
BC CT2: Health Information Exchange (p. 136)	BC CT3: Health Information Sharing Programme Management (p. 137)	Proposed Programme-level coordination supplies a plausible frame for the preserved name without replacing the established exchange abilities.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Client Administration (p. 212)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Provider Administration (p. 243)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Organisation Administration (p. 263)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Location Administration (p. 276)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Health Service Administration (p. 287)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Health Materiel Administration (p. 300)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT1: Entity Information Enablement (p. 211)	BEC CT2: Clinical Device Administration (p. 305)	Proposed This established responsibility contains materially different identity, structure, relationship or definition abilities; those become CT3s rather than treating the whole registry as a leaf.
BEC CT2: Client Administration (p. 212)	BEC CT3: Person Identity Administration (p. 214)	Proposed Identity resolution, subject context, support relationships and individual privacy are distinct constituent abilities of client representation.
BEC CT2: Client Administration (p. 212)	BEC CT3: Healthcare Subject Profile Administration (p. 226)	Proposed Identity resolution, subject context, support relationships and individual privacy are distinct constituent abilities of client representation.

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BEC CT2: Client Administration (p. 212)	BEC CT3: Client Relationship Administration (p. 232)	Proposed Identity resolution, subject context, support relationships and individual privacy are distinct constituent abilities of client representation.
BEC CT2: Client Administration (p. 212)	BEC CT3: Client Privacy Guardrail Management (p. 237)	Proposed Identity resolution, subject context, support relationships and individual privacy are distinct constituent abilities of client representation.
BEC CT2: Care Access Transaction Enablement (p. 937)	BEC CT2: Client-Centred Administration (p. 328)	Historical assertion / Proposed The responsibility concerns coordinate the intake, care-request disposition and appointment information through which a person accesses care. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Access Transaction Enablement (p. 937)	BEC CT2: Referral Administration (p. 342)	Historical assertion / Proposed The responsibility concerns coordinate the intake, care-request disposition and appointment information through which a person accesses care. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Access Transaction Enablement (p. 937)	BEC CT2: Scheduling Administration (p. 355)	Historical assertion / Proposed The responsibility concerns coordinate the intake, care-request disposition and appointment information through which a person accesses care. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Fulfilment Administration Enablement (p. 939)	BEC CT2: Episode & Encounter Administration (p. 363)	Historical assertion / Proposed The responsibility concerns maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Fulfilment Administration Enablement (p. 939)	BEC CT2: Order Administration (p. 383)	Historical assertion / Proposed The responsibility concerns maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Fulfilment Administration Enablement (p. 939)	BEC CT2: Diagnostic Administration (p. 399)	Historical assertion / Proposed The responsibility concerns maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

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BEC CT2: Care Fulfilment Administration Enablement (p. 939)	BEC CT2: Medication Administration (p. 417)	Historical assertion / Proposed The responsibility concerns maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Fulfilment Administration Enablement (p. 939)	BEC CT2: Procedure Administration (p. 435)	Historical assertion / Proposed The responsibility concerns maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Documentation & Communication Enablement (p. 940)	BEC CT2: Clinical Record Administration (p. 443)	Historical assertion / Proposed The responsibility concerns administer the lifecycle, shared-plan context and directed communication of care documentation. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Documentation & Communication Enablement (p. 940)	BEC CT2: Care Plan Administration (p. 455)	Historical assertion / Proposed The responsibility concerns administer the lifecycle, shared-plan context and directed communication of care documentation. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Documentation & Communication Enablement (p. 940)	BEC CT2: Clinical Communication Administration (p. 476)	Historical assertion / Proposed The responsibility concerns administer the lifecycle, shared-plan context and directed communication of care documentation. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Financial Information Enablement (p. 941)	BEC CT2: Funding & Coverage Administration (p. 487)	Historical assertion / Proposed The responsibility concerns represent coverage context and convey care-activity evidence to responsible funding and charging systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Financial Information Enablement (p. 941)	BEC CT2: Clinical Charging & Claims Administration (p. 489)	Historical assertion / Proposed The responsibility concerns represent coverage context and convey care-activity evidence to responsible funding and charging systems. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Specialist Clinical Context Integration (p. 945)	BEC CT2: Clinical Assessment (p. 492)	Historical assertion / Proposed The responsibility concerns integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Longitudinal & Community Care Integration (p. 942)	BEC CT3: Primary Care Coordination (p. 669)	Historical assertion / Proposed The responsibility concerns support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Facility & Episodic Care Integration (p. 943)	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Historical assertion / Proposed The responsibility concerns provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Facility & Episodic Care Integration (p. 943)	BEC CT3: Emergency Care Progression (p. 554)	Historical assertion / Proposed The responsibility concerns provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Facility & Episodic Care Integration (p. 943)	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Historical assertion / Proposed The responsibility concerns provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Facility & Episodic Care Integration (p. 943)	BEC CT2: Ambulatory Care (p. 498)	Historical assertion / Proposed The responsibility concerns provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Facility & Episodic Care Integration (p. 943)	BEC CT2: Surgical & Procedural Care (p. 499)	Historical assertion / Proposed The responsibility concerns provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Specialist Clinical Context Integration (p. 945)	BEC CT3: Diagnostic Service Integration (p. 401)	Historical assertion / Proposed The responsibility concerns integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Specialist Clinical Context Integration (p. 945)	BEC CT2: Therapeutic Services (p. 501)	Historical assertion / Proposed The responsibility concerns integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Specialist Clinical Context Integration (p. 945)	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Historical assertion / Proposed The responsibility concerns integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Specialist Clinical Context Integration (p. 945)	BEC CT2: Allied Health (p. 504)	Historical assertion / Proposed The responsibility concerns integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Longitudinal & Community Care Integration (p. 942)	BEC CT3: Screening and Immunisation Enablement (p. 329)	Historical assertion / Proposed The responsibility concerns support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Specialist Clinical Context Integration (p. 945)	BEC CT2: Rehabilitation (p. 506)	Historical assertion / Proposed The responsibility concerns integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Longitudinal & Community Care Integration (p. 942)	BEC CT3: Community Care Coordination (p. 460)	Historical assertion / Proposed The responsibility concerns support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Longitudinal & Community Care Integration (p. 942)	BEC CT3: Outreach Care Continuity (p. 466)	Historical assertion / Proposed The responsibility concerns support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Longitudinal & Community Care Integration (p. 942)	BEC CT3: Remote Patient Monitoring (p. 675)	Historical assertion / Proposed The responsibility concerns support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Setting Operations Enablement (p. 946)	BEC CT2: Clinic & Practice Operations (p. 514)	Historical assertion / Proposed The responsibility concerns coordinate the local flow, session state and capacity semantics of a care-delivery setting. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Setting Operations Enablement (p. 946)	BEC CT2: Ward Operations (p. 524)	Historical assertion / Proposed The responsibility concerns coordinate the local flow, session state and capacity semantics of a care-delivery setting. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Setting Operations Enablement (p. 946)	BEC CT2: Theatre Operations (p. 538)	Historical assertion / Proposed The responsibility concerns coordinate the local flow, session state and capacity semantics of a care-delivery setting. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Setting Operations Enablement (p. 946)	BEC CT2: Emergency Department Operations (p. 548)	Historical assertion / Proposed The responsibility concerns coordinate the local flow, session state and capacity semantics of a care-delivery setting. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Setting Operations Enablement (p. 946)	BEC CT2: Outpatient Operations (p. 561)	Historical assertion / Proposed The responsibility concerns coordinate the local flow, session state and capacity semantics of a care-delivery setting. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Resource & Capacity Coordination (p. 947)	BEC CT2: Bed & Care-Place Management (p. 569)	Historical assertion / Proposed The responsibility concerns relate operational care-place and equipment state to the coordinated view of service demand and capacity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Resource & Capacity Coordination (p. 947)	BEC CT3: Clinical Resource Management (p. 889)	Historical assertion / Proposed The responsibility concerns relate operational care-place and equipment state to the coordinated view of service demand and capacity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Care Resource & Capacity Coordination (p. 947)	BEC CT2: Service Capacity Management (p. 581)	Historical assertion / Proposed The responsibility concerns relate operational care-place and equipment state to the coordinated view of service demand and capacity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Workforce Coordination (p. 948)	BEC CT2: Clinical Workforce Capability Monitoring (p. 587)	Historical assertion / Proposed The responsibility concerns make workforce availability, coverage and eligible operational work participation usable in coordination. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Workforce Coordination (p. 948)	BEC CT2: Clinical Rostering (p. 589)	Historical assertion / Proposed The responsibility concerns make workforce availability, coverage and eligible operational work participation usable in coordination. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Workforce Coordination (p. 948)	BEC CT2: Mobile Staff Management (p. 591)	Historical assertion / Proposed The responsibility concerns make workforce availability, coverage and eligible operational work participation usable in coordination. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Workforce Coordination (p. 948)	BEC CT2: On-Call Management (p. 601)	Historical assertion / Proposed The responsibility concerns make workforce availability, coverage and eligible operational work participation usable in coordination. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Workforce Coordination (p. 948)	BEC CT2: Work Allocation & Dispatch (p. 607)	Historical assertion / Proposed The responsibility concerns make workforce availability, coverage and eligible operational work participation usable in coordination. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Workforce Qualification & Development Enablement (p. 949)	BEC CT2: Clinical Training & Education (p. 618)	Historical assertion / Proposed The responsibility concerns represent learning, competence, professional-standing checks and separately governed operational access qualifications. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Workforce Qualification & Development Enablement (p. 949)	BEC CT2: Clinical Competency Administration (p. 619)	Historical assertion / Proposed The responsibility concerns represent learning, competence, professional-standing checks and separately governed operational access qualifications. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Workforce Qualification & Development Enablement (p. 949)	BEC CT2: Clinical Qualification Administration (p. 621)	Historical assertion / Proposed The responsibility concerns represent learning, competence, professional-standing checks and separately governed operational access qualifications. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Workforce Qualification & Development Enablement (p. 949)	BEC CT2: Clinical Supervision (p. 630)	Historical assertion / Proposed The responsibility concerns represent learning, competence, professional-standing checks and separately governed operational access qualifications. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Movement & Transition Coordination (p. 950)	BEC CT2: Patient Transport (p. 632)	Historical assertion / Proposed The responsibility concerns coordinate movement of people/materials and operational departure readiness across care boundaries. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Movement & Transition Coordination (p. 950)	BEC CT2: Clinical Logistics Coordination (p. 639)	Historical assertion / Proposed The responsibility concerns coordinate movement of people/materials and operational departure readiness across care boundaries. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Care Movement & Transition Coordination (p. 950)	BEC CT2: Discharge Management (p. 645)	Historical assertion / Proposed The responsibility concerns coordinate movement of people/materials and operational departure readiness across care boundaries. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Clinical Information Stewardship Enablement (p. 952)	BEC CT2: Patient Clinical Record (p. 656)	Historical assertion / Proposed The responsibility concerns maintain governed longitudinal synthesis and the associations needed to use its constituent clinical information safely. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Health Information Connectivity Enablement (p. 953)	BEC CT2: Health Information Exchange Enablement (p. 686)	Historical assertion / Proposed The responsibility concerns make governed health information discoverable and exchangeable under explicit internal and external contracts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Health Information Connectivity Enablement (p. 953)	BEC CT2: Health Information Access (p. 701)	Historical assertion / Proposed The responsibility concerns make governed health information discoverable and exchangeable under explicit internal and external contracts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Health Information Connectivity Enablement (p. 953)	BEC CT3: Health Information Communication (p. 890)	Historical assertion / Proposed The responsibility concerns make governed health information discoverable and exchangeable under explicit internal and external contracts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Information Protection & Accountability Enablement (p. 955)	BEC CT2: Health Information Control (p. 715)	Historical assertion / Proposed The responsibility concerns apply information-use policy and preserve the accountability context needed for governed health-information activity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Semantic & Reference Knowledge Enablement (p. 954)	BEC CT2: Clinical Knowledge Services (p. 742)	Historical assertion / Proposed The responsibility concerns govern the definitions, terminology and reference content that make healthcare information consistently interpretable. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Collaborative Interaction Enablement (p. 956)	BEC CT2: Clinical Collaboration (p. 756)	Historical assertion / Proposed The responsibility concerns support contextual discussion and human interaction with governed information and activity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Collaborative Interaction Enablement (p. 956)	BEC CT2: General Collaboration (p. 768)	Historical assertion / Proposed The responsibility concerns support contextual discussion and human interaction with governed information and activity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Activity & Temporal Enablement (p. 957)	BEC CT3: Workflow & Activity Coordination (p. 891)	Historical assertion / Proposed The responsibility concerns progress governed operational work in identifiable activity and temporal contexts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Operational Activity & Temporal Enablement (p. 957)	BEC CT2: Calendar Management (p. 779)	Historical assertion / Proposed The responsibility concerns progress governed operational work in identifiable activity and temporal contexts. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.

From	To	Assertion / rationale
BEC CT2: Collaborative Interaction Enablement (p. 956)	BEC CT2: Presentation Services (p. 786)	Historical assertion / Proposed The responsibility concerns support contextual discussion and human interaction with governed information and activity. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Semantic & Reference Knowledge Enablement (p. 954)	BEC CT2: Information Design Governance (p. 792)	Historical assertion / Proposed The responsibility concerns govern the definitions, terminology and reference content that make healthcare information consistently interpretable. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Assurance Specification (p. 822)	BEC CT3: Assurance Design (p. 824)	Proposed The responsibility concerns model how satisfaction will be assured and maintain applicable, temporally qualified criteria. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Assurance Specification (p. 822)	BEC CT3: Assurance Criteria Administration (p. 828)	Proposed The responsibility concerns model how satisfaction will be assured and maintain applicable, temporally qualified criteria. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Independent Assurance Evaluation Enablement (p. 958)	BEC CT3: Governed Assurance (p. 898)	Historical assertion / Proposed The responsibility concerns associate trustworthy evidence, assess it and adjudicate independently reportable findings about governed service behaviour. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
BEC CT2: Workforce Qualification & Development Enablement (p. 949)	BEC CT3: Credential Management (p. 899)	Historical assertion / Proposed The responsibility concerns represent learning, competence, professional-standing checks and separately governed operational access qualifications. It is the finest capability frame required for the established behaviour; the older contextual heading is not another capability tier.
Enterprise CT1: Governed Information Operations (p. 837)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Governed Information Operations (p. 837)	Enterprise CT2: Context Management (p. 839)	Proposed The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Governed Information Operations (p. 837)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.

From	To	Assertion / rationale
Enterprise CT1: Governed Information Operations (p. 837)	Enterprise CT2: Information Management (p. 844)	Proposed The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Governed Information Operations (p. 837)	Enterprise CT2: Search & Discovery (p. 849)	Proposed The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Enterprise CT2: Policy & Control (p. 865)	Proposed The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Coordinated Activity & Interaction (p. 853)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Coordinated Activity & Interaction (p. 853)	Enterprise CT2: Event & Subscription (p. 856)	Proposed The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Coordinated Activity & Interaction (p. 853)	Enterprise CT2: Activity & Execution (p. 857)	Proposed The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Coordinated Activity & Interaction (p. 853)	Enterprise CT2: Interaction & Experience (p. 862)	Proposed The established reusable responsibility is a constituent ability of Coordinated Activity & Interaction; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Enterprise CT2: Operational Assurance (p. 873)	Proposed The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Governed Information Operations (p. 837)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed The established reusable responsibility is a constituent ability of Governed Information Operations; its canonical scope and exclusions remain unchanged.
Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Enterprise CT2: Service Guardian (p. 875)	Proposed The established reusable responsibility is a constituent ability of Trust, Accountability & Assurance; its canonical scope and exclusions remain unchanged.

From	To	Assertion / rationale
BEC CT2: Provider Administration (p. 243)	BEC CT3: Practitioner Profile Administration (p. 245)	Proposed Practitioner identity and professional profile information let care participants be recognised across organisations. Registration and profile standing remain attributed to the issuing authority, while discovery makes that described person usable in care integration.
BEC CT2: Provider Administration (p. 243)	BEC CT3: Practitioner Role Administration (p. 249)	Proposed The same practitioner may act in different specialties, organisations and operational capacities. Role-specific associations and communication endpoints make that participation intelligible without converting a directory role into permission or professional privilege.
BEC CT2: Provider Administration (p. 243)	BEC CT3: Provider Relationship (p. 925)	Historical assertion / Proposed Effective affiliations explain which practitioner or team participates with which organisation. These relationships support collaboration and routing while each participating identity keeps its own responsibility owner.
BEC CT2: Provider Administration (p. 243)	BEC CT3: Provider Engagement (p. 926)	Historical assertion / Proposed Engagement information represents the terms and externally granted practice privileges under which a provider participates. This differs from who the practitioner is and from whether a particular operation is authorised now.
BEC CT2: Organisation Administration (p. 263)	BEC CT3: Healthcare Organisation Profile Administration (p. 265)	Proposed An identifiable legal or operational healthcare organisation needs verified identifiers, a profile and published standing. This description anchors service and affiliation relationships without managing the organisation itself.
BEC CT2: Organisation Administration (p. 263)	BEC CT3: Organisational Structure (p. 927)	Historical assertion / Proposed Departments, divisions and regional networks need qualified structural relationships that can be traversed and understood over time. Structural membership is distinct from physical location and from authority to manage another organisation.
BEC CT2: Organisation Administration (p. 263)	BEC CT3: Organisation Contact (p. 928)	Historical assertion / Proposed Verified facility contact channels make an organisation reachable for appropriate administrative and clinical communication. The contact record says how it may be reached; recipient qualification and disclosure policy govern a particular use.
BEC CT2: Location Administration (p. 276)	BEC CT3: Healthcare Location Profile Administration (p. 277)	Proposed Addresses, geospatial context and facility identity establish which physical place an information or activity concern refers to. Describing a location remains separate from deciding what services can operate there now.

From	To	Assertion / rationale
BEC CT2: Location Administration (p. 276)	BEC CT3: Location Structure (p. 930)	Historical assertion / Proposed Campus, building, floor, ward and room relationships explain physical containment. The structure provides navigation and context without determining organisational reporting lines or current operational capacity.
BEC CT2: Location Administration (p. 276)	BEC CT3: Care-Place Definition Administration (p. 281)	Proposed A particular bed, bay, treatment chair or procedural place has stable physical characteristics relevant to care. Those descriptive capabilities remain separate from occupancy, cleaning state and suitability for a current patient.
BEC CT2: Health Service Administration (p. 287)	BEC CT3: Healthcare Service Profile Administration (p. 289)	Proposed An offered service has an identifiable clinical or administrative purpose, specialty and published intake description. That definition is distinct from the provider/location binding through which the service can be delivered.
BEC CT2: Health Service Administration (p. 287)	BEC CT3: Service Provision Relationship (p. 931)	Historical assertion / Proposed Service provision joins a named offering to the organisations, places and practitioner roles through which it is provided. The mapping is the owned concern; the identity and meaning of each mapped participant remain with its established capability.
BEC CT2: Health Service Administration (p. 287)	BEC CT3: Service Availability Definition (p. 932)	Historical assertion / Proposed Regular hours, on-call windows and exceptional closures describe when an offering is nominally available. They provide an input to readiness and scheduling without proving that all required dependencies are ready.
BEC CT2: Health Service Administration (p. 287)	BEC CT3: Service Eligibility Definition (p. 933)	Historical assertion / Proposed Published catchment, referral and clinical eligibility conditions explain the intended audience and access constraints of an offering. Their representation supports responsible assessment without deciding acceptance or clinical priority.
BEC CT2: Health Materiel Administration (p. 300)	BEC CT3: Health Materiel Profile Administration (p. 301)	Proposed Medication, blood-product, implant and consumable descriptions provide reference context for care information. Product identity and classification are retained without turning this reference responsibility into procurement or prescribing.
BEC CT2: Health Materiel Administration (p. 300)	BEC CT3: Health Materiel Catalogue & Formulary Administration (p. 302)	Proposed An approved facility formulary or product catalogue provides a qualified set of products relevant to local care. Representing its membership and source standing remains distinct from deciding formulary approval and from generic terminology governance.
BEC CT2: Health Materiel Administration (p. 300)	BEC CT3: Health Materiel Traceability Administration (p. 304)	Proposed Batch, lot and serial references make a product instance or supply-chain association traceable when care information needs it. This reference scope does not run inventory, recall operations or the supply chain.

From	To	Assertion / rationale
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Clinical Device Profile Administration (p. 307)	Proposed A device type has a model, regulatory description and stated capabilities used to interpret equipment context. This type-level responsibility differs from identifying the actual physical instance that produced information.
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Device Instance (p. 934)	Historical assertion / Proposed An identifiable physical device carries UDI and serial context through its use. The instance record supports association and resolution without originating device observations or making maintenance decisions.
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Device Association (p. 935)	Historical assertion / Proposed A device can be associated with different patients and care places at different times. Qualified association and its lineage allow observations to be interpreted against the applicable binding rather than the current assignment alone.
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Device Connectivity (p. 936)	Historical assertion / Proposed A device communication binding identifies the protocol and endpoint through which its feed can participate. Connectivity metadata supports routing without owning authentication issuance, payload semantics or physiological interpretation.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Care Access Transaction Enablement (p. 937)	Historical assertion / Proposed Coordinate the intake, care-request disposition and appointment information through which a person accesses care. Its constituent abilities distinguish Client-Centred Administration; Referral Administration; Scheduling Administration.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Care Fulfilment Administration Enablement (p. 939)	Historical assertion / Proposed Maintain distinguishable care-episode, bounded fulfilment and delivered-activity information across performing systems. Its constituent abilities distinguish Episode & Encounter Administration; Order Administration; Diagnostic Administration; Medication Administration; Procedure Administration.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Care Documentation & Communication Enablement (p. 940)	Historical assertion / Proposed Administer the lifecycle, shared-plan context and directed communication of care documentation. Its constituent abilities distinguish Clinical Record Administration; Care Plan Administration; Clinical Communication Administration.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Care Financial Information Enablement (p. 941)	Historical assertion / Proposed Represent coverage context and convey care-activity evidence to responsible funding and charging systems. Its constituent abilities distinguish Funding & Coverage Administration; Clinical Charging & Claims Administration.

From	To	Assertion / rationale
BEC CT1: Care Context Integration Enablement (p. 923)	BEC CT2: Longitudinal & Community Care Integration (p. 942)	Historical assertion / Proposed Support continuity, remote participation and preventive follow-up through appropriately assembled and exchanged care context. Its constituent abilities distinguish Primary Care; Community Care; Outreach Care; Remote & Virtual Care; Preventive Care.
BEC CT1: Care Context Integration Enablement (p. 923)	BEC CT2: Facility & Episodic Care Integration (p. 943)	Historical assertion / Proposed Provide the context assemblies and boundary integration needed for facility presentations and bounded care episodes. Its constituent abilities distinguish Acute Care; Emergency Care; Inpatient Care; Ambulatory Care; Surgical & Procedural Care.
BEC CT1: Care Context Integration Enablement (p. 923)	BEC CT2: Specialist Clinical Context Integration (p. 945)	Historical assertion / Proposed Integrate specialist assessment, investigation and therapeutic information with longitudinal and episode context. Its constituent abilities distinguish Clinical Assessment; Diagnostic Services; Therapeutic Services; Medication Therapy; Allied Health; Rehabilitation.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Care Setting Operations Enablement (p. 946)	Historical assertion / Proposed Coordinate the local flow, session state and capacity semantics of a care-delivery setting. Its constituent abilities distinguish Clinic & Practice Operations; Ward Operations; Theatre Operations; Emergency Department Operations; Outpatient Operations.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Care Resource & Capacity Coordination (p. 947)	Historical assertion / Proposed Relate operational care-place and equipment state to the coordinated view of service demand and capacity. Its constituent abilities distinguish Bed & Care-Place Management; Clinical Resource Management; Service Capacity Management.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Operational Workforce Coordination (p. 948)	Historical assertion / Proposed Make workforce availability, coverage and eligible operational work participation usable in coordination. Its constituent abilities distinguish Clinical Workforce Management; Clinical Rostering; Mobile Staff Management; On-Call Management; Work Allocation & Dispatch.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Workforce Qualification & Development Enablement (p. 949)	Historical assertion / Proposed Represent learning, competence, professional-standing checks and separately governed operational access qualifications. Its constituent abilities distinguish Clinical Training & Education; Clinical Competency Management; Clinical Qualification Management; Clinical Supervision; Credential Management.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Care Movement & Transition Coordination (p. 950)	Historical assertion / Proposed Coordinate movement of people/materials and operational departure readiness across care boundaries. Its constituent abilities distinguish Patient Transport; Clinical Logistics Coordination; Discharge Management.

From	To	Assertion / rationale
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Clinical Information Stewardship Enablement (p. 952)	Historical assertion / Proposed Maintain governed longitudinal synthesis and the associations needed to use its constituent clinical information safely. Its constituent abilities distinguish Patient Clinical Record.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Health Information Connectivity Enablement (p. 953)	Historical assertion / Proposed Make governed health information discoverable and exchangeable under explicit internal and external contracts. Its constituent abilities distinguish Health Information Exchange; Health Information Access; Health Information Communication.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Semantic & Reference Knowledge Enablement (p. 954)	Historical assertion / Proposed Govern the definitions, terminology and reference content that make healthcare information consistently interpretable. Its constituent abilities distinguish Clinical Knowledge Services; Information Design Governance.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Information Protection & Accountability Enablement (p. 955)	Historical assertion / Proposed Apply information-use policy and preserve the accountability context needed for governed health-information activity. Its constituent abilities distinguish Health Information Control.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT1: Health Informatics Management (p. 811)	Historical assertion / Proposed Provide existing health information for authorised research, required population notification and service planning purposes. Its constituent abilities distinguish .
BEC CT1: Human Collaboration & Work Enablement (p. 924)	BEC CT2: Collaborative Interaction Enablement (p. 956)	Historical assertion / Proposed Support contextual discussion and human interaction with governed information and activity. Its constituent abilities distinguish Clinical Collaboration; General Collaboration; Presentation Services.
BEC CT1: Human Collaboration & Work Enablement (p. 924)	BEC CT2: Operational Activity & Temporal Enablement (p. 957)	Historical assertion / Proposed Progress governed operational work in identifiable activity and temporal contexts. Its constituent abilities distinguish Workflow & Activity Coordination; Calendar Management.
BEC CT1: Independent Assurance Enablement (p. 821)	BEC CT2: Assurance Specification (p. 822)	Proposed Model how satisfaction will be assured and maintain applicable, temporally qualified criteria. Its constituent abilities distinguish Assurance Design; Assurance Criteria Management.
BEC CT1: Independent Assurance Enablement (p. 821)	BEC CT2: Independent Assurance Evaluation Enablement (p. 958)	Historical assertion / Proposed Associate trustworthy evidence, assess it and adjudicate independently reportable findings about governed service behaviour. Its constituent abilities distinguish Governed Assurance.
BEC CT2: Clinical Information Stewardship Enablement (p. 952)	BEC CT3: Linked Information Curation (p. 663)	Historical assertion / Proposed Logical information may refer to one or several payloads held internally or by an external custodian. This capability keeps association and custody knowledge explicit so that content can be obtained under the governing information contract.

From	To	Assertion / rationale
BEC CT2: Clinical Information Stewardship Enablement (p. 952)	BEC CT3: Information Authority Representation (p. 959)	Historical assertion / Proposed A persisted assertion may be authoritative, informational or anecdotal. This ability carries that standing into interpretation without treating transport reliability, freshness or successful commit as proof of credibility.
BEC CT2: Semantic & Reference Knowledge Enablement (p. 954)	BEC CT3: Governed Reference Dataset Administration (p. 748)	Historical assertion / Proposed Value-set membership, mapping content and governed nomenclature may change independently of the structural definition that gives them meaning. Updates need identifiable review and effective standing without redefining model libraries as mutable data.
BEC CT2: Information Protection & Accountability Enablement (p. 955)	BEC CT3: Significant Activity Audit Capture (p. 961)	Historical assertion / Proposed Mandatory evidence capture has its own policy-governed platform authority and must remain possible when the audited identity is denied an action. The captured fact is distinguished from debug output and the identity whose activity is recorded.
BEC CT2: Information Protection & Accountability Enablement (p. 955)	BEC CT3: Data Provenance Capture (p. 733)	Historical assertion / Proposed Recipients need to know which assertions were received, transformed or superseded and why. This ability preserves those meaningful relationships without making every retry or active-state reconstruction permanent provenance.
BEC CT1: Health Informatics Management (p. 811)	BEC CT2: Research Information Curation (p. 812)	Proposed Research support must retain the purpose, applicable approvals and release qualification of supplied information. It reuses governed retrieval and privacy controls rather than claiming a clinical-trial or research-governance platform.
BEC CT1: Health Informatics Management (p. 811)	BEC CT2: Population Health Information Reporting (p. 817)	Proposed A reportable event and the obligation to notify are different from sending a message. This capability preserves the established obligation and observes the notification outcome under its contract without deciding public-health response.
BEC CT1: Health Informatics Management (p. 811)	BEC CT2: Service Planning Data Management (p. 818)	Proposed Planners need comparable time- and service-qualified observations from multiple operating settings. This ability prepares those projections while leaving the interpretation of required future capacity and distribution with the planning authority.
BEC CT2: Information Protection & Accountability Enablement (p. 955)	BEC CT3: Governance Requirement Publication Support (p. 962)	Historical assertion / Proposed Consumers need an identifiable statement of which rule applies and who established it. This proposal fills BC-17 information support without giving the platform authority to make legislation, approve governance requirements or reinterpret their force.

From	To	Assertion / rationale
BEC CT2: Operational Activity & Temporal Enablement (p. 957)	BEC CT3: Entity-Specific Operational Coordination (p. 963)	Historical assertion / Proposed Activity concerning the same patient, practitioner or bed needs a coherent entity-relative frame. Interest fan-out, serial progression, fresh information context and uncertain effects remain distinguishable from generic multi-work-unit progression.
BEC CT2: Operational Activity & Temporal Enablement (p. 957)	BEC CT3: Governed Operational Activity Design (p. 964)	Historical assertion / Proposed An activity needs a fulfilment definition even when only one bounded execution unit is involved. This capability specifies its semantic envelope while execution and event delivery remain separate responsibilities.
BEC CT2: Care Resource & Capacity Coordination (p. 947)	BEC CT3: Service Offering Readiness Coordination (p. 966)	Historical assertion / Proposed Collective readiness depends on suitable workforce, spaces, equipment, materials and supporting services. This proposal evaluates their joint availability for an offering without treating one available bed, an operating-hours declaration or aggregate capacity as service readiness.
BC CT2: Clinical Determination (p. 10)	BC CT3: Clinical Finding Interpretation (p. 11)	Proposed Interpret observed clinical findings and their significance for the individual care concern. The proposed decomposition separates this outcome from the other responsibilities within Clinical Determination.
BC CT2: Clinical Determination (p. 10)	BC CT3: Diagnosis & Problem Determination (p. 12)	Proposed Establish and qualify the clinical problems or diagnoses applicable to an individual. The proposed decomposition separates this outcome from the other responsibilities within Clinical Determination.
BC CT2: Clinical Determination (p. 10)	BC CT3: Health Risk & Prognostic Determination (p. 13)	Proposed Establish clinically meaningful risk and expected-course judgements for an individual. The proposed decomposition separates this outcome from the other responsibilities within Clinical Determination.
BC CT2: Individual Care Planning (p. 14)	BC CT3: Care Goal Definition (p. 16)	Proposed Establish the intended outcomes and priorities against which an individual care response is planned. The proposed decomposition separates this outcome from the other responsibilities within Individual Care Planning.
BC CT2: Individual Care Planning (p. 14)	BC CT3: Care Response Planning (p. 17)	Proposed Determine the intended clinical response and its sequencing in relation to agreed care goals. The proposed decomposition separates this outcome from the other responsibilities within Individual Care Planning.
BC CT2: Medication & Pharmacy Services (p. 58)	BC CT3: Medication Compounding & Preparation (p. 63)	Proposed Prepare or compound a clinically required medication formulation under the applicable professional and product-governance mandate. The proposed decomposition separates this outcome from the other responsibilities within Medication & Pharmacy Services.

From	To	Assertion / rationale
BC CT1: Care Access & Coordination (p. 26)	BC CT2: Service Delivery Preparedness Management (p. 44)	Proposed Preparedness asks whether the service as a whole can operate, distinct from scheduling capacity or readiness of a single place. This is a proposed missing business frame for the already identified readiness concern.
Enterprise CT2: Information Management (p. 844)	Enterprise CT3: Active Information Availability (p. 846)	Proposed Active representation, observation and convergence support availability while preserving an explicit trust basis for use.
Enterprise CT2: Information Management (p. 844)	Enterprise CT3: Authoritative Durable State Establishment (p. 847)	Proposed Durable acceptance gives managed information an explicit committed state from which applicable active representations can be reconstructed.
Enterprise CT2: Information Management (p. 844)	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed Document attachments, diagnostic media and operational artefacts share a need to locate usable content while distinguishing logical meaning from physical possession.
Enterprise CT2: Policy & Control (p. 865)	Enterprise CT3: Operation Policy Resolution (p. 867)	Proposed Permission depends on controlled criteria, role assignments, action and information scope; revocation is distinct from natural expiry.
Enterprise CT2: Policy & Control (p. 865)	Enterprise CT3: Operation-Bound Policy Enforcement (p. 868)	Proposed Distributed participants apply the applicable effective authorisation and fail closed when authority is absent or untrustworthy.
Enterprise CT2: Provenance & Traceability (p. 869)	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed Lineage explains how an assertion reached its present form without turning every infrastructure event into provenance.
Enterprise CT2: Provenance & Traceability (p. 869)	Enterprise CT3: Governed Activity Audit Evidence (p. 872)	Proposed Denied actions and successful mutations may both require trustworthy evidence; the audited identity and capture authority remain distinct.
Enterprise CT2: Activity & Execution (p. 857)	Enterprise CT3: Operational Fulfilment Definition (p. 859)	Proposed The semantic execution envelope applies to one-step and multi-step activity and stays distinct from dispatch and supervision.
Enterprise CT2: Activity & Execution (p. 857)	Enterprise CT3: Governed Activity Execution Control (p. 860)	Proposed Reusable execution control respects work-unit archetype, participation and uncertain effects rather than equating dispatch with completion.
Enterprise CT2: Activity & Execution (p. 857)	Enterprise CT3: Entity-Relative Operational Progression (p. 861)	Proposed Patient, practitioner and bed concerns recur across different business contexts; each can need a coherent progression frame without one central entity service.
Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed Identity correlation, clinical-history synthesis, reference-content use and assurance evidence all need to distinguish the standing of an assertion from technical acceptance. This recurring ability is insufficiently explicit in the existing fourteen EC definitions and is proposed as a new peer responsibility.

From	To	Assertion / rationale
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Client-Centred Administration (p. 328)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Referral Administration (p. 342)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Scheduling Administration (p. 355)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Episode & Encounter Administration (p. 363)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Order Administration (p. 383)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Diagnostic Administration (p. 399)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Medication Administration (p. 417)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Procedure Administration (p. 435)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Clinical Record Administration (p. 443)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Care Plan Administration (p. 455)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Clinical Communication Administration (p. 476)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Funding & Coverage Administration (p. 487)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Care Administration Enablement (p. 326)	BEC CT2: Clinical Charging & Claims Administration (p. 489)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Clinical Assessment (p. 492)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Patient Clinical Record (p. 656)	BEC CT3: Primary Care Coordination (p. 669)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Episode & Encounter Administration (p. 363)	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Emergency Department Operations (p. 548)	BEC CT3: Emergency Care Progression (p. 554)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Ward Operations (p. 524)	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Ambulatory Care (p. 498)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Surgical & Procedural Care (p. 499)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT2: Diagnostic Administration (p. 399)	BEC CT3: Diagnostic Service Integration (p. 401)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Therapeutic Services (p. 501)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Medication Administration (p. 417)	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Allied Health (p. 504)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Client-Centred Administration (p. 328)	BEC CT3: Screening and Immunisation Enablement (p. 329)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Rehabilitation (p. 506)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Care Plan Administration (p. 455)	BEC CT3: Community Care Coordination (p. 460)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Care Plan Administration (p. 455)	BEC CT3: Outreach Care Continuity (p. 466)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Patient Clinical Record (p. 656)	BEC CT3: Remote Patient Monitoring (p. 675)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinic & Practice Operations (p. 514)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Ward Operations (p. 524)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Theatre Operations (p. 538)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Emergency Department Operations (p. 548)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Outpatient Operations (p. 561)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Bed & Care-Place Management (p. 569)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Service Capacity Management (p. 581)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Workforce Capability Monitoring (p. 587)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Rostering (p. 589)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Mobile Staff Management (p. 591)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: On-Call Management (p. 601)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Work Allocation & Dispatch (p. 607)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Training & Education (p. 618)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Competency Administration (p. 619)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Qualification Administration (p. 621)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Supervision (p. 630)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Patient Transport (p. 632)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Clinical Logistics Coordination (p. 639)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Discharge Management (p. 645)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Patient Clinical Record (p. 656)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Health Information Exchange Enablement (p. 686)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Health Information Access (p. 701)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Health Information Control (p. 715)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Clinical Collaboration (p. 756)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: General Collaboration (p. 768)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Calendar Management (p. 779)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Presentation Services (p. 786)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Information Design Governance (p. 792)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Patient Clinical Record (p. 656)	BEC CT3: Linked Information Curation (p. 663)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Knowledge Services (p. 742)	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Content Maintainer Management (p. 731)	BEC CT3: Data Provenance Capture (p. 733)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Provider Administration (p. 243)	BEC CT3: Vendor Liaison (p. 253)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Device Utilisation Administration (p. 311)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Device OA&M Metadata Management (p. 316)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Device Administration (p. 305)	BEC CT3: Device Connectivity Compliance Management (p. 319)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Referral Administration (p. 342)	BEC CT3: Referral Intake & Progression (p. 344)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Scheduling Administration (p. 355)	BEC CT3: Appointment & Booking Administration (p. 357)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Order Administration (p. 383)	BEC CT3: Closed-Loop Order Progression (p. 385)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Record Administration (p. 443)	BEC CT3: Clinical Documentation Lifecycle Administration (p. 445)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Care Plan Administration (p. 455)	BEC CT3: Care Plan Lifecycle & Coordination (p. 457)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Primary Care (p. 494)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Acute Care (p. 495)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Emergency Care (p. 496)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Inpatient Care (p. 497)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Diagnostic Services (p. 500)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Medication Therapy (p. 503)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Preventive Care (p. 505)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Community Care (p. 508)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Outreach Care (p. 509)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Clinical Service Dependency Management (p. 491)	BEC CT2: Remote & Virtual Care (p. 510)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinic & Practice Operations (p. 514)	BEC CT3: Session & Queue Coordination (p. 515)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Ward Operations (p. 524)	BEC CT3: Ward Logistics Coordination (p. 525)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Theatre Operations (p. 538)	BEC CT3: Perioperative Logistics Coordination (p. 540)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Emergency Department Operations (p. 548)	BEC CT3: Emergency Department Operational Coordination (p. 549)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Outpatient Operations (p. 561)	BEC CT3: Outpatient Flow Coordination (p. 563)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Bed & Care-Place Management (p. 569)	BEC CT3: Bed & Care-Place State Administration (p. 571)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Bed & Care-Place Management (p. 569)	BEC CT3: Bed & Care-Place Turnover Coordination (p. 573)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Service Capacity Management (p. 581)	BEC CT3: Service Demand & Capacity Coordination (p. 582)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Service Establishment Support (p. 586)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Rostering (p. 589)	BEC CT3: Clinical Roster Capture & Communication (p. 590)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Mobile Staff Management (p. 591)	BEC CT3: Mobile Worker Dispatch & Safety Coordination (p. 593)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: On-Call Management (p. 601)	BEC CT3: On-Call Availability & Contact Resolution (p. 603)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Work Allocation & Dispatch (p. 607)	BEC CT3: Operational Work Assignment & Dispatch Coordination (p. 608)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Qualification Administration (p. 621)	BEC CT3: Organisation Clinical Certification & Compliance Support (p. 622)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Qualification Administration (p. 621)	BEC CT3: Provider Clinical Qualification Tracking (p. 623)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT1: Health Service Operations Enablement (p. 512)	BEC CT2: Credential Management (p. 627)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Credential Management (p. 627)	BEC CT3: Clinical Service Credential Definition Management (p. 629)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Supervision (p. 630)	BEC CT3: Clinical Escalation Path Management (p. 631)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Patient Transport (p. 632)	BEC CT3: Patient Transfer Coordination (p. 634)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Logistics Coordination (p. 639)	BEC CT3: Specimen Metadata Management (p. 641)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Logistics Coordination (p. 639)	BEC CT3: Specimen Movement Traceability (p. 642)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Discharge Management (p. 645)	BEC CT3: Discharge Progression & Coordination (p. 647)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Patient Clinical Record (p. 656)	BEC CT3: Longitudinal Clinical Record Administration (p. 658)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Exchange Enablement (p. 686)	BEC CT3: HIE Requirement Management (p. 688)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Exchange Enablement (p. 686)	BEC CT3: HIE Implementation Management (p. 689)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Exchange Enablement (p. 686)	BEC CT3: HIE Endpoint State Management (p. 694)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Access (p. 701)	BEC CT3: Governed Health Information Retrieval (p. 703)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Health Information Communication (p. 710)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Communication (p. 710)	BEC CT3: Patient Notification Services (p. 711)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Communication (p. 710)	BEC CT3: Patient Task Communication Services (p. 712)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Communication (p. 710)	BEC CT3: Patient Self-Service (p. 713)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Communication (p. 710)	BEC CT3: Patient Health Library Access (p. 714)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Control (p. 715)	BEC CT3: Patient Health Information Restriction Management (p. 718)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Health Information Control (p. 715)	BEC CT3: Patient Health Information Filter Management (p. 720)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT2: Health Information Control (p. 715)	BEC CT3: Patient Health Information Disclosure Management (p. 721)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Information Asset Stewardship Management (p. 728)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Asset Stewardship Management (p. 728)	BEC CT3: Information Asset Definition (p. 729)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Asset Stewardship Management (p. 728)	BEC CT3: Information Asset Tag Management (p. 730)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Information Content Maintainer Management (p. 731)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Content Maintainer Management (p. 731)	BEC CT3: Data Scientist Administration (p. 732)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Content Maintainer Management (p. 731)	BEC CT3: Information Product Administration (p. 737)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Knowledge Services (p. 742)	BEC CT3: Clinical Knowledge Request Administration (p. 744)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Knowledge Services (p. 742)	BEC CT3: Clinical Knowledge Artefact Distribution Management (p. 747)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Collaboration (p. 756)	BEC CT3: Clinical Context Collaboration (p. 758)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: General Collaboration (p. 768)	BEC CT3: Operational Collaboration (p. 770)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Shared Information & Coordination Enablement (p. 654)	BEC CT2: Workflow & Activity Coordination (p. 773)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Workflow & Activity Coordination (p. 773)	BEC CT3: External Task Coordination (p. 774)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Workflow & Activity Coordination (p. 773)	BEC CT3: External Task Status & Traceability Management (p. 775)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Workflow & Activity Coordination (p. 773)	BEC CT3: Activity Routing Management (p. 777)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Workflow & Activity Coordination (p. 773)	BEC CT3: Work Boundary Governance (p. 778)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Calendar Management (p. 779)	BEC CT3: Operational & Clinical Calendar Coordination (p. 780)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Presentation Services (p. 786)	BEC CT3: Contextual Information Presentation (p. 788)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

From	To	Assertion / rationale
BEC CT2: Information Design Governance (p. 792)	BEC CT3: Enterprise Canonical/Reusable Model Design (p. 794)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Design Governance (p. 792)	BEC CT3: Enterprise Canonical/Reusable Model Library Management (p. 799)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Design Governance (p. 792)	BEC CT3: Enterprise Canonical/Reusable Model Utilisation Support (p. 800)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Information Design Governance (p. 792)	BEC CT3: Enterprise Reference Data Stewardship (p. 802)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT1: Independent Assurance Enablement (p. 821)	BEC CT2: Assurance Service Review (p. 832)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Assurance Service Review (p. 832)	BEC CT3: Assurance Service Evaluation (p. 834)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Client-Centred Administration (p. 328)	BEC CT3: Client Intake & Demographic Update (p. 336)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Episode & Encounter Administration (p. 363)	BEC CT3: Encounter Lifecycle Management (p. 371)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Diagnostic Administration (p. 399)	BEC CT3: Diagnostic Workflow Management (p. 408)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Medication Administration (p. 417)	BEC CT3: Medication Order & Dispense Management (p. 426)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Procedure Administration (p. 435)	BEC CT3: Procedure & Surgical Administration (p. 437)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.
BEC CT2: Clinical Communication Administration (p. 476)	BEC CT3: Secure Clinical Communication (p. 478)	Proposed Human-reviewed candidate hierarchy; structural relationship remains Proposed.

32.2. Realises Feature

From	To	Assertion / rationale
Function: Resolve Person Identifier (p. 219)	Feature: Identifier Resolution (p. 216)	Established em:24 (p. 1435)
Function: Correlate Person Identifiers (p. 220)	Feature: Cross-Authority Identifier Correlation (p. 217)	Established em:27 (p. 1435)
Function: Maintain Person Identity Aliases (p. 222)	Feature: Identity Alias Association (p. 218)	Established em:30 (p. 1435)
Function: Apply Governed Person Identity Correction (p. 222)	Feature: Governed Identity Correction (p. 218)	Established em:33 (p. 1435)
Function: Establish Healthcare Subject Context (p. 229)	Feature: Subject Context Binding (p. 227)	Established em:45 (p. 1436)
Function: Govern Subject Demographic Context (p. 230)	Feature: Demographic Context Governance (p. 228)	Established em:48 (p. 1436)

From	To	Assertion / rationale
Function: Maintain Client Relationships (p. 235)	Feature: Next-of-Kin & Guardian Association (p. 233)	Established em:59 (p. 1436)
Function: Govern Client Relationship Validity (p. 236)	Feature: Relationship Validity Tracking (p. 234)	Established em:62 (p. 1436)
Function: Evaluate Client Consent (p. 240)	Feature: Consent Directive Evaluation (p. 239)	Established em:73 (p. 1437)
Function: Evaluate Client Confidentiality Restrictions (p. 242)	Feature: Confidentiality Flag Enforcement (p. 240)	Established em:76 (p. 1437)
Function: Verify Practitioner Registration (p. 254)	Feature: National Practitioner Verification (p. 246)	Established em:92 (p. 1437)
Function: Govern Practitioner Profile (p. 256)	Feature: Practitioner Profile Governance (p. 247)	Established em:96 (p. 1437)
Function: Maintain Practitioner Roles (p. 257)	Feature: Role & Specialty Modeling (p. 251)	Established em:99 (p. 1437)
Function: Bind Practitioner Electronic Endpoints (p. 258)	Feature: Electronic Endpoint Binding (p. 252)	Established em:102 (p. 1437)
Function: Maintain Practitioner Affiliations (p. 260)	Feature: Practitioner-Organisation Affiliation (p. 252)	Established em:105 (p. 1437)
Function: Maintain Clinical Privileges (p. 261)	Feature: Clinical Privileging Record (p. 625)	Established em:108 (p. 1437)
Function: Verify Healthcare Organisation (p. 270)	Feature: National Organisation Verification (p. 267)	Established em:121 (p. 1438)
Function: Govern Organisation Profile (p. 272)	Feature: Organisation Profile Management (p. 268)	Established em:124 (p. 1438)
Function: Maintain Organisational Structure (p. 273)	Feature: Hierarchical Structure Navigation (p. 268)	Established em:127 (p. 1438)
Function: Maintain Organisation Contacts (p. 274)	Feature: Directory Contact Management (p. 269)	Established em:130 (p. 1438)
Function: Maintain Healthcare Location (p. 283)	Feature: Location Geospatial & Address Resolution (p. 279)	Established em:145 (p. 1439)
Function: Maintain Physical Location Hierarchy (p. 284)	Feature: Physical Hierarchy Modeling (p. 280)	Established em:148 (p. 1439)
Function: Maintain Care-Place Definition (p. 286)	Feature: Atomic Care-Place Modeling (p. 282)	Established em:151 (p. 1439)
Function: Maintain Healthcare Service Definition (p. 294)	Feature: Service Catalogue Publishing (p. 291)	Established em:174 (p. 1439)
Function: Maintain Service / Location / Provider Map (p. 296)	Feature: Service-Location-Provider Binding (p. 291)	Established em:177 (p. 1439)
Function: Maintain Service Availability (p. 297)	Feature: Operating Schedule Specification (p. 292)	Established em:180 (p. 1439)
Function: Maintain Service Eligibility Rules (p. 298)	Feature: Eligibility & Catchment Specification (p. 293)	Established em:183 (p. 1439)

From	To	Assertion / rationale
Function: Maintain Device Definition (p. 320)	Feature: Device Type Specification (p. 309)	Established em:196 (p. 1440)
Function: Maintain Device Instance Identity (p. 321)	Feature: Device Serial Tracking (p. 310)	Established em:199 (p. 1440)
Function: Maintain Device Association (p. 322)	Feature: Device-Patient/Location Assignment (p. 312)	Established em:202 (p. 1440)
Function: Maintain Device Communication Endpoints (p. 323)	Feature: Device Endpoint Registration (p. 318)	Established em:205 (p. 1440)
Function: Receive Client Demographic Change (p. 339)	Feature: Demographic Update Ingestion (p. 337)	Established sa:18 (p. 1441)
Function: Distribute Governed Demographic Change (p. 341)	Feature: Demographic Change Distribution (p. 338)	Established sa:21 (p. 1441)
Function: Receive Referral (p. 349)	Feature: Referral Document Ingestion (p. 346)	Established sa:33 (p. 1441)
Function: Assemble Referral Context (p. 351)	Feature: Referral Supporting Information (p. 346)	Established sa:36 (p. 1441)
Function: Manage Referral Progression (p. 352)	Feature: Referral Status & Outcome Tracking (p. 347)	Established sa:39 (p. 1441)
Function: Receive Appointment Notification (p. 360)	Feature: Appointment Notification Ingestion (p. 358)	Established sa:53 (p. 1442)
Function: Synchronise Appointment Status (p. 361)	Feature: Appointment Status Synchronization (p. 359)	Established sa:56 (p. 1442)
Function: Establish Encounter Context (p. 377)	Feature: Encounter Context Creation (p. 373)	Established sa:68 (p. 1442)
Function: Progress Encounter State (p. 378)	Feature: Encounter State Progression (p. 374)	Established sa:71 (p. 1442)
Function: Maintain Encounter Care-Place Association (p. 380)	Feature: Encounter Bed/Location Association (p. 375)	Established sa:76 (p. 1442)
Function: Receive Order Request (p. 391)	Feature: Order Request Ingestion (p. 386)	Established sa:89 (p. 1443)
Function: Resolve Order Destination (p. 392)	Feature: Order Destination Resolution & Routing (p. 387)	Established sa:92 (p. 1443)
Function: Manage Order Progression (p. 393)	Feature: Order Closed-Loop Progression Tracking (p. 388)	Established sa:95 (p. 1443)
Function: Coordinate Order Modification / Cancellation (p. 395)	Feature: Order Cancellation & Modification Coordination (p. 389)	Established sa:100 (p. 1443)
Function: Correlate Diagnostic Request (p. 413)	Feature: Diagnostic Request Correlation (p. 409)	Established sa:116 (p. 1444)
Function: Bind Diagnostic Report (p. 414)	Feature: Diagnostic Report Ingestion & Binding (p. 410)	Established sa:119 (p. 1444)
Function: Distribute Diagnostic Report (p. 416)	Feature: Diagnostic Report Distribution (p. 411)	Established sa:122 (p. 1444)
Function: Receive Medication Order (p. 431)	Feature: Medication Order Ingestion (p. 428)	Established sa:136 (p. 1444)

From	To	Assertion / rationale
Function: Track Dispense Event (p. 432)	Feature: Dispense Event Tracking (p. 429)	Established sa:139 (p. 1444)
Function: Receive Medication Administration Event (p. 434)	Feature: Medication Administration Record (MAR) Ingestion (p. 430)	Established sa:142 (p. 1444)
Function: Receive Procedure Booking / Request (p. 440)	Feature: Procedure Booking Ingestion (p. 438)	Established sa:153 (p. 1445)
Function: Receive Procedural Documentation (p. 442)	Feature: Procedural Documentation Ingestion (p. 439)	Established sa:156 (p. 1445)
Function: Receive Clinical Document (p. 450)	Feature: Clinical Document Ingestion (p. 447)	Established sa:168 (p. 1445)
Function: Govern Clinical Document Lifecycle (p. 451)	Feature: Document Versioning & Supersession (p. 448)	Established sa:171 (p. 1445)
Function: Index Clinical Document Metadata (p. 452)	Feature: Document Metadata Indexing (p. 449)	Established sa:176 (p. 1445)
Function: Receive Care Plan (p. 472)	Feature: Care Plan Ingestion & Distribution (p. 458)	Established sa:186 (p. 1446)
Function: Distribute Clinical Communication (p. 482)	Feature: Clinical Communication Distribution (p. 479)	Established sa:206 (p. 1446)
Function: Track Business Delivery Acknowledgement (p. 484)	Feature: Delivery Acknowledgement Tracking (p. 480)	Established sa:209 (p. 1446)
Function: Record Communication Transaction (p. 485)	Feature: Communication Audit Logging (p. 481)	Established sa:213 (p. 1446)
Function: Correlate Primary Care Consultation Context (p. 672)	Feature: General Practice Shared Record Ingestion (p. 671)	Established sd:55 (p. 1449)
Function: Assemble Acute Clinical Baseline (p. 368)	Feature: Acute Care Clinical Context Provision (p. 367)	Established sd:75 (p. 1450)
Function: Bind ED Triage Assessment (p. 557)	Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	Established sd:95 (p. 1450)
Function: Provide Emergency LHR Summary (p. 558)	Feature: Emergency Care Cross-Facility Correlation (p. 556)	Established sd:98 (p. 1450)
Function: Consolidate Inpatient Clinical Round Feed (p. 533)	Feature: Inpatient Rounding Context Provision (p. 531)	Established sd:114 (p. 1451)
Function: Project Longitudinal Medication History (p. 423)	Feature: Medication History Reconciliation Ingestion (p. 421)	Established sd:151 (p. 1452)
Function: Syndicate Immunisation Record (p. 333)	Feature: Immunisation Event Submission (p. 331)	Established sd:168 (p. 1452)
Function: Synchronize Community Visit Record (p. 463)	Feature: Community Encounter Ingestion (p. 462)	Established sd:187 (p. 1453)
Function: Provide Outreach Visit Context (p. 470)	Feature: Outreach Visit Context Provision (p. 468)	Established sd:206 (p. 1453)

From	To	Assertion / rationale
Function: Reconcile Outreach Clinical Batch (p. 470)	Feature: Outreach Encounter Outcome Ingestion (p. 469)	Established sd:210 (p. 1453)
Function: Correlate Virtual Care Consultation Context (p. 679)	Feature: Virtual Care Session Context Binding (p. 678)	Established sd:225 (p. 1454)
Function: Ingest Remote Patient Telemetry (p. 680)	Feature: Remote Telemetry Ingestion (p. 677)	Established sd:228 (p. 1454)
Function: Track Clinic Operating Session (p. 520)	Feature: Clinic Session Tracking (p. 517)	Established hso:78 (p. 1457)
Function: Manage Clinic Queue Progression (p. 521)	Feature: Operational Queue Progression (p. 518)	Established hso:82 (p. 1457)
Function: Assemble Ward Handover Context (p. 537)	Feature: Clinical Handover Context Assembly (p. 528)	Established hso:99 (p. 1457)
Function: Manage Theatre Case Progression (p. 545)	Feature: Operating Theatre Case Progression (p. 541)	Established hso:112 (p. 1458)
Function: Capture Post-Clinic Follow-up Requirements (p. 569)	Feature: Post-Clinic Order Coordination (p. 565)	Established hso:144 (p. 1459)
Function: Maintain Operational Bed State (p. 576)	Feature: Bed Availability Tracking (p. 572)	Established hso:157 (p. 1459)
Function: Manage Bed Turnover Progression (p. 578)	Feature: Bed State Progression (p. 575)	Established hso:160 (p. 1459)
Function: Track Mobile Clinical Asset Operational State (p. 314)	Feature: Mobile Clinical Equipment Tracking (p. 313)	Established hso:172 (p. 1460)
Function: Aggregate Service Capacity Telemetry (p. 585)	Feature: Operational Capacity Metric Aggregation (p. 584)	Established hso:182 (p. 1460)
Function: Receive Mobile Visit Progression (p. 600)	Feature: Mobile Visit Status Tracking (p. 595)	Established hso:200 (p. 1460)
Function: Integrate Mobile Work Management (p. 601)	Feature: Mobile Worker Task Dispatch (p. 594)	Established hso:203 (p. 1460)
Function: Resolve Active On-Call Coverage (p. 605)	Feature: Active On-Call Provider Resolution (p. 604)	Established hso:214 (p. 1461)
Function: Dispatch Operational Work Order (p. 614)	Feature: Work Item Instantiation (p. 610)	Established hso:227 (p. 1462)
Function: Manage Operational Work Progression (p. 615)	Feature: Work Progress Oversight & Escalation (p. 613)	Established hso:231 (p. 1462)
Function: Manage Patient Transport Progression (p. 637)	Feature: Transport Dispatch & Progress Tracking (p. 636)	Established hso:253 (p. 1462)
Function: Manage Specimen Transport Progression (p. 902)	Feature: Pathology Specimen Transport Tracking (p. 643)	Historical assertion / Established hso:267 (p. 1463)
Function: Manage Discharge Coordination Progression (p. 651)	Feature: Discharge Readiness & Coordination Oversight (p. 649)	Established hso:279 (p. 1463)
Function: Verify Clinical Qualification for Work Dispatch (p. 627)	Feature: Operational Credential Check (p. 626)	Established hso:295 (p. 1464)

From	To	Assertion / rationale
Function: Assemble Longitudinal Clinical Record (p. 681)	Feature: Longitudinal Health Record (LHR) Assembly (p. 659)	Established ise:39 (p. 1466)
Function: Maintain Active Clinical Record (p. 683)	Feature: Active Clinical Record Access (p. 660)	Established ise:42 (p. 1466)
Function: Preserve Durable Clinical Record (p. 684)	Feature: Durable Clinical Record Preservation (p. 661)	Established ise:45 (p. 1466)
Function: Receive Information Submission (p. 695)	Feature: Direct Submission Ingestion & Delivery (p. 690)	Established ise:57 (p. 1466)
Function: Mediate Information Retrieval (p. 697)	Feature: Clinical Record Retrieval Mediation (p. 691)	Established ise:60 (p. 1466)
Function: Distribute Addressed Information (p. 698)	Feature: Addressed Clinical Distribution (p. 692)	Established ise:63 (p. 1466)
Function: Syndicate Information Change (p. 700)	Feature: Event-Driven Syndication (p. 693)	Established ise:66 (p. 1466)
Function: Search Health Information (p. 706)	Feature: Federated Clinical Query (p. 704)	Established ise:78 (p. 1467)
Function: Mediate Standards-Based Health Information Communication (p. 902)	Feature: Standards-Based Boundary Exchange (p. 892)	Historical assertion / Established ise:97 (p. 1467)
Function: Evaluate Information Access Authority (p. 722)	Feature: Health Information Access & Consent Control (p. 719)	Established ise:108 (p. 1468)
Function: Record Security-Significant Activity (p. 727)	Feature: Non-PHI Compliance Auditing (p. 894)	Historical assertion / Established ise:119 (p. 1468)
Function: Resolve Clinical Concept (p. 753)	Feature: Clinical Concept Resolution (p. 745)	Established ise:130 (p. 1468)
Function: Map Clinical Concept (p. 755)	Feature: Bidirectional Terminology Mapping (p. 746)	Established ise:133 (p. 1468)
Function: Coordinate Care-Team Collaboration (p. 763)	Feature: Care-Team Clinical Collaboration Coordination (p. 759)	Established ise:146 (p. 1469)
Function: Resolve LHR Query within Collaboration (p. 765)	Feature: In-Conversation LHR Query Resolution (p. 760)	Established ise:149 (p. 1469)
Function: Govern Collaboration Metadata (p. 766)	Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	Established ise:154 (p. 1469)
Function: Coordinate Operational Collaboration (p. 772)	Feature: Operational Team Chat Enablement (p. 771)	Established ise:164 (p. 1469)
Function: Coordinate Work Order (p. 904)	Feature: Work Order Progression (p. 895)	Historical assertion / Established ise:181 (p. 1469)
Function: Coordinate To Do (p. 905)	Feature: To Do Decision & Approval Coordination (p. 895)	Historical assertion / Established ise:185 (p. 1469)
Function: Coordinate Synthetic Task (p. 907)	Feature: Synthetic Task Orchestration (p. 896)	Historical assertion / Established ise:189 (p. 1469)
Function: Supervise Activity Timeout / Escalation (p. 908)	Feature: Workflow Timeout & Escalation Oversight (p. 897)	Historical assertion / Established ise:193 (p. 1469)

From	To	Assertion / rationale
Function: Project Operational Schedule (p. 783)	Feature: Schedule Projection Integration (p. 781)	Established ise:205 (p. 1470)
Function: Correlate Activity with Temporal Context (p. 785)	Feature: Temporal Event Correlation (p. 782)	Established ise:208 (p. 1470)
Function: Present Contextual Health and Operational Information (p. 790)	Feature: Longitudinal Clinical & Operational Presentation (p. 789)	Established ise:220 (p. 1471)
Function: Govern Information Semantics (p. 803)	Feature: Information Standards & Semantic Governance (p. 795)	Established ise:231 (p. 1471)
Function: Verify Semantic Conformance (p. 808)	Feature: Semantic Conformance Verification (p. 801)	Established ise:243 (p. 1471)
Function: Manage Theatre Case Progression (p. 545)	Feature: Perioperative Resource Notification (p. 542)	Established hso:114 (p. 1458)
Function: Coordinate ED Departmental Capacity (p. 560)	Feature: ED Bed & Bay Tracking (p. 551)	Established hso:127 (p. 1458)
Function: Manage Outpatient Capacity (p. 567)	Feature: Outpatient Arrival Registration (p. 564)	Established hso:139 (p. 1459)
Function: Manage Bed Turnover Progression (p. 578)	Feature: Bed Readiness Notification (p. 576)	Established hso:162 (p. 1459)
Function: Dispatch Operational Work Order (p. 614)	Feature: Worker Matching & Allocation (p. 611)	Established hso:229 (p. 1462)
Function: Dispatch Operational Work Order (p. 614)	Feature: Work Dispatch Delivery (p. 612)	Established hso:229 (p. 1462)
Function: Manage Patient Transport Progression (p. 637)	Feature: Transport Request Ingestion (p. 635)	Established hso:256 (p. 1462)
Function: Manage Discharge Coordination Progression (p. 651)	Feature: Discharge Milestone Tracking (p. 648)	Established hso:280 (p. 1463)
Function: Manage Discharge Coordination Progression (p. 651)	Feature: Discharge Notification Dispatch (p. 650)	Established hso:281 (p. 1463)
Function: Coordinate Multi-Agency Chronic Care Plan (p. 674)	Feature: Primary Care Notification Dispatch (p. 671)	Unresolved; not an asserted association sd:58 (p. 1449)
Function: Route Acute Critical Result Alert (p. 370)	Feature: Acute Clinical State Event Capture (p. 366)	Unresolved; not an asserted association sd:78 (p. 1450)
Function: Assemble Inpatient Discharge Dossier (p. 534)	Feature: Inpatient Clinical Progression Tracking (p. 532)	Unresolved; not an asserted association sd:117 (p. 1451)
Function: Receive Diagnostic Requisition Feed (p. 404)	Feature: Laboratory & Imaging Result Distribution (p. 403)	Unresolved; not an asserted association sd:133 (p. 1451)
Function: Publish Governed Diagnostic Report (p. 406)	Feature: Diagnostic History Consolidation (p. 404)	Unresolved; not an asserted association sd:137 (p. 1451)

From	To	Assertion / rationale
Function: Correlate Adverse Reaction & Allergy Profile (p. 424)	Feature: Medication Administration Tracking (p. 422)	Unresolved; not an asserted association sd:154 (p. 1452)
Function: Project Screening Eligibility Timeline (p. 334)	Feature: Screening Recall Notification Distribution (p. 332)	Unresolved; not an asserted association sd:171 (p. 1452)
Function: Bind Community Support Network Context (p. 465)	Feature: Community Care Plan Synchronization (p. 463)	Unresolved; not an asserted association sd:190 (p. 1453)
Function: Track Operational Staff Presence (p. 598)	Feature: Mobile Worker Task Dispatch (p. 594)	Unresolved; not an asserted association hso:196 (p. 1460)
Function: Retrieve Health Information (p. 708)	Feature: Access-Controlled Information Filtering (p. 705)	Unresolved; not an asserted association ise:81 (p. 1467)
Function: Evaluate Consent Constraint (p. 724)	Feature: Security Context Propagation (p. 893)	Historical assertion / Unresolved; not an asserted association ise:111 (p. 1468)
Function: Associate Order Outcome (p. 396)	Feature: Order Outcome Association (p. 390)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Govern Information Definition (p. 804)	Feature: Information Definition & Schema Governance (p. 796)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Govern Terminology Binding (p. 806)	Feature: Terminology Binding Governance (p. 797)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Govern Exchange Profile (p. 807)	Feature: Exchange Profile Governance (p. 798)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Design Service Assurance (p. 826)	Feature: Assurance Disposition & Design Modelling (p. 828)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Manage Assurance Criteria (p. 830)	Feature: Temporal Assurance Criteria Stewardship (p. 831)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Establish Assurance Context (p. 909)	Feature: Independent Evidence Assessment & Adjudication (p. 982)	Historical assertion / Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Assess Assurance Evidence (p. 911)	Feature: Independent Evidence Assessment & Adjudication (p. 982)	Historical assertion / Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Adjudicate Assurance Assessment (p. 912)	Feature: Independent Evidence Assessment & Adjudication (p. 982)	Historical assertion / Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Manage Clinic & Practice Capacity (p. 520)	Feature: Clinic Session Capacity Management (p. 519)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.

From	To	Assertion / rationale
Function: Manage Ward Capacity (p. 536)	Feature: Ward Care Capacity Management (p. 528)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Manage Theatre Capacity (p. 544)	Feature: Theatre Session Capacity Management (p. 543)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Coordinate ED Departmental Capacity (p. 560)	Feature: Emergency Treatment Capacity Management (p. 553)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Manage Outpatient Capacity (p. 567)	Feature: Outpatient Appointment Capacity Management (p. 566)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Manage Mobile Service Capacity (p. 597)	Feature: Mobile Service Capacity Management (p. 596)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.
Function: Resolve Associated Binary Content (p. 666)	Feature: Binary Content Resolution & Assembly (p. 665)	Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Assemble Associated Binary Content (p. 667)	Feature: Binary Content Resolution & Assembly (p. 665)	Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Qualify Information Authority Standing (p. 983)	Feature: Information Authority & Source Standing Representation (p. 967)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Accept Governed Dataset Revision (p. 751)	Feature: Governed Dataset Content Revision (p. 750)	Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Capture Required Significant Audit Evidence (p. 984)	Feature: Mandatory Significant Activity Audit Capture (p. 968)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Record Significant Information Lineage (p. 736)	Feature: Information Transformation Lineage (p. 735)	Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Coordinate Entity-Relative Input (p. 986)	Feature: Entity-Relative Activity Coordination (p. 973)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Maintain Operational Fulfilment Definition (p. 988)	Feature: Operational Activity Context Definition (p. 975)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Establish Activity Effect Observation Basis (p. 991)	Feature: Uncertain Activity Outcome Reconciliation (p. 977)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Reconcile Indeterminate Activity Effects (p. 990)	Feature: Uncertain Activity Outcome Reconciliation (p. 977)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.

From	To	Assertion / rationale
Function: Determine Reconciled Activity Continuation (p. 992)	Feature: Uncertain Activity Outcome Reconciliation (p. 977)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Assess Service Offering Readiness (p. 994)	Feature: Service Readiness Dependency Assessment (p. 979)	Historical assertion / Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.
Function: Assemble Purpose-Qualified Research Release (p. 814)	Feature: Authorised Research Release Assembly (p. 739)	Proposed The proposed state change materially realises this Feature; this is a graph edge, not an ownership chain.

32.3. Associated Feature

From	To	Assertion / rationale
Service: Person Identifier Resolution (p. 220)	Feature: Identifier Resolution (p. 216)	Established em:25 (p. 1435)
Service: Person Identifier Correlation (p. 221)	Feature: Cross-Authority Identifier Correlation (p. 217)	Established em:28 (p. 1435)
Service: Person Identity Correction (p. 223)	Feature: Governed Identity Correction (p. 218)	Established em:34 (p. 1435)
Service: Healthcare Subject Context Resolution (p. 230)	Feature: Subject Context Binding (p. 227)	Established em:46 (p. 1436)
Service: Demographic Change Distribution (p. 231)	Feature: Demographic Context Governance (p. 228)	Established em:49 (p. 1436)
Service: Client Relationship Query (p. 235)	Feature: Next-of-Kin & Guardian Association (p. 233)	Established em:60 (p. 1436)
Service: Representative Authority Verification (p. 236)	Feature: Relationship Validity Tracking (p. 234)	Established em:63 (p. 1436)
Service: Consent Decision Service (p. 241)	Feature: Consent Directive Evaluation (p. 239)	Established em:74 (p. 1437)
Service: Confidentiality Restriction Evaluation (p. 242)	Feature: Confidentiality Flag Enforcement (p. 240)	Established em:77 (p. 1437)
Service: Practitioner Verification Service (p. 255)	Feature: National Practitioner Verification (p. 246)	Established em:93 (p. 1437)
Service: Practitioner Directory Query (p. 256)	Feature: Practitioner Profile Governance (p. 247)	Established em:97 (p. 1437)
Service: Practitioner Role Resolution (p. 258)	Feature: Role & Specialty Modeling (p. 251)	Established em:100 (p. 1437)
Service: Practitioner Endpoint Lookup (p. 259)	Feature: Electronic Endpoint Binding (p. 252)	Established em:103 (p. 1437)
Service: Practitioner Affiliation Resolution (p. 260)	Feature: Practitioner-Organisation Affiliation (p. 252)	Established em:106 (p. 1437)
Service: Clinical Privilege Verification (p. 262)	Feature: Clinical Privileging Record (p. 625)	Established em:109 (p. 1437)
Service: Organisation Verification Service (p. 271)	Feature: National Organisation Verification (p. 267)	Established em:122 (p. 1438)

From	To	Assertion / rationale
Service: Organisation Directory Query (p. 272)	Feature: Organisation Profile Management (p. 268)	Established em:125 (p. 1438)
Service: Organisational Hierarchy Resolution (p. 274)	Feature: Hierarchical Structure Navigation (p. 268)	Established em:128 (p. 1438)
Service: Organisation Contact Lookup (p. 275)	Feature: Directory Contact Management (p. 269)	Established em:131 (p. 1438)
Service: Location Directory Query (p. 284)	Feature: Location Geospatial & Address Resolution (p. 279)	Established em:146 (p. 1439)
Service: Location Hierarchy Resolution (p. 285)	Feature: Physical Hierarchy Modeling (p. 280)	Established em:149 (p. 1439)
Service: Care-Place Specification Lookup (p. 286)	Feature: Atomic Care-Place Modeling (p. 282)	Established em:152 (p. 1439)
Service: Healthcare Service Directory Query (p. 295)	Feature: Service Catalogue Publishing (p. 291)	Established em:175 (p. 1439)
Service: Service Provision Resolution (p. 296)	Feature: Service-Location-Provider Binding (p. 291)	Established em:178 (p. 1439)
Service: Service Availability Query (p. 298)	Feature: Operating Schedule Specification (p. 292)	Established em:181 (p. 1439)
Service: Service Eligibility Evaluation (p. 299)	Feature: Eligibility & Catchment Specification (p. 293)	Established em:184 (p. 1439)
Service: Device Definition Query (p. 320)	Feature: Device Type Specification (p. 309)	Established em:197 (p. 1440)
Service: Device Instance Resolution (p. 322)	Feature: Device Serial Tracking (p. 310)	Established em:200 (p. 1440)
Service: Device Association Query (p. 323)	Feature: Device-Patient/Location Assignment (p. 312)	Established em:203 (p. 1440)
Service: Device Endpoint Resolution (p. 324)	Feature: Device Endpoint Registration (p. 318)	Established em:206 (p. 1440)
Service: Demographic Intake Service (p. 340)	Feature: Demographic Update Ingestion (p. 337)	Established sa:19 (p. 1441)
Service: Demographic Update Notification (p. 342)	Feature: Demographic Change Distribution (p. 338)	Established sa:22 (p. 1441)
Service: Referral Submission Service (p. 350)	Feature: Referral Document Ingestion (p. 346)	Established sa:34 (p. 1441)
Service: Referral Context Query (p. 352)	Feature: Referral Supporting Information (p. 346)	Established sa:37 (p. 1441)
Service: Referral Status & Outcome Service (p. 353)	Feature: Referral Status & Outcome Tracking (p. 347)	Established sa:40 (p. 1441)
Service: Appointment Notification Ingress (p. 360)	Feature: Appointment Notification Ingestion (p. 358)	Established sa:54 (p. 1442)
Service: Appointment Schedule Query (p. 362)	Feature: Appointment Status Synchronization (p. 359)	Established sa:57 (p. 1442)
Service: Encounter Context Resolution (p. 378)	Feature: Encounter Context Creation (p. 373)	Established sa:69 (p. 1442)

From	To	Assertion / rationale
Service: Encounter Lifecycle Event Notification (p. 379)	Feature: Encounter State Progression (p. 374)	Established sa:72 (p. 1442)
Service: Encounter Location History Query (p. 381)	Feature: Encounter Bed/Location Association (p. 375)	Established sa:77 (p. 1442)
Service: Order Requisition Ingress (p. 391)	Feature: Order Request Ingestion (p. 386)	Established sa:90 (p. 1443)
Service: Order Dispatch Service (p. 393)	Feature: Order Destination Resolution & Routing (p. 387)	Established sa:93 (p. 1443)
Service: Order Status & Tracking Query (p. 394)	Feature: Order Closed-Loop Progression Tracking (p. 388)	Established sa:96 (p. 1443)
Service: Order Cancellation Service (p. 396)	Feature: Order Cancellation & Modification Coordination (p. 389)	Established sa:101 (p. 1443)
Service: Diagnostic Correlation Resolution (p. 414)	Feature: Diagnostic Request Correlation (p. 409)	Established sa:117 (p. 1444)
Service: Diagnostic Report Ingress (p. 415)	Feature: Diagnostic Report Ingestion & Binding (p. 410)	Established sa:120 (p. 1444)
Service: Diagnostic Result Delivery Service (p. 417)	Feature: Diagnostic Report Distribution (p. 411)	Established sa:124 (p. 1444)
Service: Medication Order Ingress (p. 431)	Feature: Medication Order Ingestion (p. 428)	Established sa:137 (p. 1444)
Service: Dispense Event Ingress (p. 433)	Feature: Dispense Event Tracking (p. 429)	Established sa:140 (p. 1444)
Service: Medication Administration Ingress (p. 434)	Feature: Medication Administration Record (MAR) Ingestion (p. 430)	Established sa:143 (p. 1444)
Service: Procedure Booking Ingress (p. 441)	Feature: Procedure Booking Ingestion (p. 438)	Established sa:154 (p. 1445)
Service: Procedural Documentation Ingress (p. 443)	Feature: Procedural Documentation Ingestion (p. 439)	Established sa:157 (p. 1445)
Service: Clinical Document Ingress (p. 450)	Feature: Clinical Document Ingestion (p. 447)	Established sa:169 (p. 1445)
Service: Clinical Document Lifecycle Service (p. 452)	Feature: Document Versioning & Supersession (p. 448)	Established sa:172 (p. 1445)
Service: Document Metadata Registry Query (p. 453)	Feature: Document Metadata Indexing (p. 449)	Established sa:177 (p. 1445)
Service: Care Plan Ingress (p. 472)	Feature: Care Plan Ingestion & Distribution (p. 458)	Established sa:187 (p. 1446)
Service: Secure Clinical Message Dispatch (p. 483)	Feature: Clinical Communication Distribution (p. 479)	Established sa:207 (p. 1446)
Service: Communication Delivery Status Query (p. 485)	Feature: Delivery Acknowledgement Tracking (p. 480)	Established sa:211 (p. 1446)
Service: Communication Audit Query (p. 486)	Feature: Communication Audit Logging (p. 481)	Established sa:214 (p. 1446)

From	To	Assertion / rationale
Service: Primary Care Summary Ingress (p. 673)	Feature: General Practice Shared Record Ingestion (p. 671)	Established sd:56 (p. 1449)
Service: Acute Admission Dossier Query (p. 369)	Feature: Acute Care Clinical Context Provision (p. 367)	Established sd:76 (p. 1450)
Service: ED Triage Stream Ingress (p. 558)	Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	Established sd:96 (p. 1450)
Service: Emergency Fast-Path LHR Query (p. 559)	Feature: Emergency Care Cross-Facility Correlation (p. 556)	Established sd:99 (p. 1450)
Service: Ward Round Clinical Briefing Query (p. 534)	Feature: Inpatient Rounding Context Provision (p. 531)	Established sd:115 (p. 1451)
Service: Longitudinal Medication History Query (p. 424)	Feature: Medication History Reconciliation Ingestion (p. 421)	Established sd:152 (p. 1452)
Service: Immunisation History Query (p. 334)	Feature: Immunisation Event Submission (p. 331)	Established sd:169 (p. 1452)
Service: Community Care Encounter Ingress (p. 464)	Feature: Community Encounter Ingestion (p. 462)	Established sd:188 (p. 1453)
Service: Outreach Batch Reconciliation Service (p. 471)	Feature: Outreach Encounter Outcome Ingestion (p. 469)	Established sd:211 (p. 1453)
Service: Virtual Encounter Context Resolution (p. 679)	Feature: Virtual Care Session Context Binding (p. 678)	Established sd:226 (p. 1454)
Service: Remote Telemetry Ingress (p. 681)	Feature: Remote Telemetry Ingestion (p. 677)	Established sd:229 (p. 1454)
Service: Clinic Queue Telemetry Service (p. 522)	Feature: Operational Queue Progression (p. 518)	Established hso:83 (p. 1457)
Service: Theatre Case Progression Telemetry (p. 546)	Feature: Operating Theatre Case Progression (p. 541)	Established hso:113 (p. 1458)
Service: Bed Availability Query (p. 577)	Feature: Bed Availability Tracking (p. 572)	Established hso:158 (p. 1459)
Service: Bed Turnover Lifecycle Service (p. 579)	Feature: Bed State Progression (p. 575)	Established hso:161 (p. 1459)
Service: Clinical Resource Availability Query (p. 315)	Feature: Mobile Clinical Equipment Tracking (p. 313)	Established hso:173 (p. 1460)
Service: Regional Health System Capacity Query (p. 585)	Feature: Operational Capacity Metric Aggregation (p. 584)	Established hso:183 (p. 1460)
Service: On-Call Specialist Lookup (p. 606)	Feature: Active On-Call Provider Resolution (p. 604)	Established hso:215 (p. 1461)
Service: Work Dispatch Service (p. 614)	Feature: Work Item Instantiation (p. 610)	Established hso:228 (p. 1462)
Service: Operational Work Status Query (p. 616)	Feature: Work Progress Oversight & Escalation (p. 613)	Established hso:232 (p. 1462)
Service: Patient Transport Tracking Service (p. 638)	Feature: Transport Dispatch & Progress Tracking (p. 636)	Established hso:254 (p. 1462)

From	To	Assertion / rationale
Service: Specimen Transit Tracking Service (p. 644)	Feature: Pathology Specimen Transport Tracking (p. 643)	Established hso:268 (p. 1463)
Service: Discharge Readiness Telemetry (p. 652)	Feature: Discharge Readiness & Coordination Oversight (p. 649)	Established hso:283 (p. 1463)
Service: Longitudinal Clinical Record Query (p. 682)	Feature: Longitudinal Health Record (LHR) Assembly (p. 659)	Established ise:40 (p. 1466)
Service: Active Problem & Allergy Summary Query (p. 684)	Feature: Active Clinical Record Access (p. 660)	Established ise:43 (p. 1466)
Service: Historical Clinical Record Access (p. 685)	Feature: Durable Clinical Record Preservation (p. 661)	Established ise:46 (p. 1466)
Service: HIE Submission Gateway Service (p. 696)	Feature: Direct Submission Ingestion & Delivery (p. 690)	Established ise:58 (p. 1466)
Service: Federated Health Information Query (p. 698)	Feature: Clinical Record Retrieval Mediation (p. 691)	Established ise:61 (p. 1466)
Service: Addressed Document Delivery Service (p. 699)	Feature: Addressed Clinical Distribution (p. 692)	Established ise:64 (p. 1466)
Service: Clinical Event Syndication Stream (p. 700)	Feature: Event-Driven Syndication (p. 693)	Established ise:67 (p. 1466)
Service: Clinical Information Search Service (p. 707)	Feature: Federated Clinical Query (p. 704)	Established ise:79 (p. 1467)
Service: Standards Communication Gateway Service (p. 903)	Feature: Standards-Based Boundary Exchange (p. 892)	Historical assertion / Established ise:98 (p. 1467)
Service: Policy Evaluation & Authorisation Service (p. 723)	Feature: Health Information Access & Consent Control (p. 719)	Established ise:109 (p. 1468)
Service: Security Audit Ingress (p. 727)	Feature: Non-PHI Compliance Auditing (p. 894)	Historical assertion / Established ise:120 (p. 1468)
Service: Terminology Concept Resolution Service (p. 754)	Feature: Clinical Concept Resolution (p. 745)	Established ise:131 (p. 1468)
Service: Terminology Mapping Service (p. 755)	Feature: Bidirectional Terminology Mapping (p. 746)	Established ise:134 (p. 1468)
Service: Clinical Collaboration Space Service (p. 764)	Feature: Care-Team Clinical Collaboration Coordination (p. 759)	Established ise:147 (p. 1469)
Service: Collaboration Clinical Summary Resolution (p. 766)	Feature: In-Conversation LHR Query Resolution (p. 760)	Established ise:150 (p. 1469)
Service: Collaboration Space Governance Service (p. 767)	Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	Established ise:155 (p. 1469)
Service: Operational Collaboration Service (p. 772)	Feature: Operational Team Chat Enablement (p. 771)	Established ise:165 (p. 1469)
Service: Work Order Coordination Service (p. 905)	Feature: Work Order Progression (p. 895)	Historical assertion / Established ise:182 (p. 1469)

From	To	Assertion / rationale
Service: To Do Management Service (p. 906)	Feature: To Do Decision & Approval Coordination (p. 895)	Historical assertion / Established ise:186 (p. 1469)
Service: Synthetic Task Execution Service (p. 907)	Feature: Synthetic Task Orchestration (p. 896)	Historical assertion / Established ise:190 (p. 1469)
Service: Activity Escalation Telemetry (p. 909)	Feature: Workflow Timeout & Escalation Oversight (p. 897)	Historical assertion / Established ise:194 (p. 1469)
Service: Unified Calendar Projection Query (p. 784)	Feature: Schedule Projection Integration (p. 781)	Established ise:206 (p. 1470)
Service: Temporal Context Resolution (p. 785)	Feature: Temporal Event Correlation (p. 782)	Established ise:209 (p. 1470)
Service: Contextual Presentation Service (p. 791)	Feature: Longitudinal Clinical & Operational Presentation (p. 789)	Established ise:221 (p. 1471)
Service: Semantic Model Definition Service (p. 804)	Feature: Information Standards & Semantic Governance (p. 795)	Established ise:232 (p. 1471)
Service: Semantic Conformance Verification Service (p. 809)	Feature: Semantic Conformance Verification (p. 801)	Established ise:244 (p. 1471)
Service: Chronic Care Timeline Query (p. 674)	Feature: Primary Care Notification Dispatch (p. 671)	Unresolved; not an asserted association sd:58 (p. 1449)
Service: Acute Clinical Alert Dispatch (p. 370)	Feature: Acute Clinical State Event Capture (p. 366)	Unresolved; not an asserted association sd:78 (p. 1450)
Service: Discharge Dossier Packaging Service (p. 535)	Feature: Inpatient Clinical Progression Tracking (p. 532)	Unresolved; not an asserted association sd:117 (p. 1451)
Service: Modality Worklist Distribution (p. 405)	Feature: Laboratory & Imaging Result Distribution (p. 403)	Unresolved; not an asserted association sd:133 (p. 1451)
Service: Diagnostic Publication Service (p. 407)	Feature: Diagnostic History Consolidation (p. 404)	Unresolved; not an asserted association sd:137 (p. 1451)
Service: Allergy & Adverse Reaction Query (p. 425)	Feature: Medication Administration Tracking (p. 422)	Unresolved; not an asserted association sd:154 (p. 1452)
Service: Preventive Screening Status Query (p. 335)	Feature: Screening Recall Notification Distribution (p. 332)	Unresolved; not an asserted association sd:171 (p. 1452)
Service: Community Care Team Query (p. 466)	Feature: Community Care Plan Synchronization (p. 463)	Unresolved; not an asserted association sd:190 (p. 1453)
Service: Staff Presence Telemetry Query (p. 599)	Feature: Mobile Worker Task Dispatch (p. 594)	Unresolved; not an asserted association hso:196 (p. 1460)
Service: Clinical Resource Retrieval Service (p. 709)	Feature: Access-Controlled Information Filtering (p. 705)	Unresolved; not an asserted association ise:81 (p. 1467)
Service: Consent Enforcement Service (p. 724)	Feature: Security Context Propagation (p. 893)	Historical assertion / Unresolved; not an asserted association ise:111 (p. 1468)
Service: Practitioner Directory Query (p. 256)	Feature: Provider Directory Discovery & Resolution (p. 248)	Proposed This existing behaviour materially discharges the proposed Feature; ownership and canonical definition remain unchanged.

32.4. Participating Function

From	To	Assertion / rationale
Process: Assurance Process Design (p. 827)	Function: Design Service Assurance (p. 826)	Established processes:361 (p. 1422)
Process: Assurance Process Design (p. 827)	Function: Manage Assurance Criteria (p. 830)	Established processes:361 (p. 1422)
Process: Assurance Process Execution (p. 920)	Function: Establish Assurance Context (p. 909)	Historical assertion / Established processes:373 (p. 1422)
Process: Assurance Process Execution (p. 920)	Function: Assess Assurance Evidence (p. 911)	Historical assertion / Established processes:373 (p. 1422)
Process: Assurance Process Execution (p. 920)	Function: Adjudicate Assurance Assessment (p. 912)	Historical assertion / Established processes:373 (p. 1422)
Process: Indeterminate Activity Outcome Reconciliation Process (p. 993)	Function: Reconcile Indeterminate Activity Effects (p. 990)	Historical assertion / Proposed Reconciliation supplies effect qualification within the process; remaining observation and authorisation use existing responsibilities.
Process: Indeterminate Activity Outcome Reconciliation Process (p. 993)	Function: Establish Activity Effect Observation Basis (p. 991)	Historical assertion / Proposed This Function participates in the explicitly ordered proposed reconciliation chain, retaining its capability responsibility.
Process: Indeterminate Activity Outcome Reconciliation Process (p. 993)	Function: Determine Reconciled Activity Continuation (p. 992)	Historical assertion / Proposed This Function participates in the explicitly ordered proposed reconciliation chain, retaining its capability responsibility.

32.5. Responsibility Parent

From	To	Assertion / rationale
BEC CT2: Client Administration (p. 212)	BEC CT3: Person Identity Administration (p. 214)	Established em:16 (p. 1435)

32.6. Contributes Feature

From	To	Assertion / rationale
Function: Manage Ward Capacity (p. 536)	Feature: Ward Patient Flow Coordination (p. 527)	Established hso:96 (p. 1457)

32.7. Enabled By

From	To	Assertion / rationale
BC CT1: Individual Care Delivery (p. 5)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)
BC CT2: Individual Health Assessment (p. 6)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)
BC CT2: Clinical Determination (p. 10)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)

From	To	Assertion / rationale
BC CT2: Individual Care Planning (p. 14)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)
BC CT2: Clinical Intervention (p. 18)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)
BC CT2: Individual Health Monitoring (p. 19)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)
BC CT2: Individual Care Review (p. 22)	BEC CT2: Health Information Access (p. 701)	Established business:314 (p. 1335)
BC CT1: Health Service Assurance (p. 202)	BEC CT3: Assurance Design (p. 824)	Established enabling:676 (p. 1382)
BC CT1: Health Service Assurance (p. 202)	BEC CT3: Assurance Criteria Administration (p. 828)	Established enabling:676 (p. 1382)
BC CT1: Health Service Assurance (p. 202)	BEC CT3: Governed Assurance (p. 898)	Historical assertion / Established enabling:676 (p. 1382)
BC CT1: Individual Care Delivery (p. 5)	BEC CT2: Patient Clinical Record (p. 656)	Proposed Care decisions require qualified longitudinal information and the representation of externally agreed plans; professional authority remains with care actors. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BC CT1: Individual Care Delivery (p. 5)	BEC CT2: Care Plan Administration (p. 455)	Proposed Care decisions require qualified longitudinal information and the representation of externally agreed plans; professional authority remains with care actors. Examples: Feature: Care Plan Ingestion & Distribution; Function: Receive Care Plan; Service: Care Plan Ingress; Function: Associate Care Plan Context; Service: Care Plan Context Query; Function: Distribute Care Plan Change
BC CT2: Care Access Management (p. 27)	BEC CT2: Referral Administration (p. 342)	Proposed Intake, referral disposition and booking synchronisation support access without deciding acceptance of clinical responsibility. Examples: Feature: Referral Document Ingestion; Feature: Referral Supporting Information; Feature: Referral Status & Outcome Tracking; Function: Receive Referral; Service: Referral Submission Service; Function: Assemble Referral Context

From	To	Assertion / rationale
BC CT2: Care Access Management (p. 27)	BEC CT2: Scheduling Administration (p. 355)	Proposed Intake, referral disposition and booking synchronisation support access without deciding acceptance of clinical responsibility. Examples: Feature: Appointment Notification Ingestion; Feature: Appointment Status Synchronization; Function: Receive Appointment Notification; Service: Appointment Notification Ingress; Function: Synchronise Appointment Status; Service: Appointment Schedule Query
BC CT2: Care Access Management (p. 27)	BEC CT2: Client-Centred Administration (p. 328)	Proposed Intake, referral disposition and booking synchronisation support access without deciding acceptance of clinical responsibility. Examples: Feature: Demographic Update Ingestion; Feature: Demographic Change Distribution; Function: Receive Client Demographic Change; Service: Demographic Intake Service; Function: Distribute Governed Demographic Change; Service: Demographic Update Notification
BC CT2: Care Scheduling & Capacity Management (p. 31)	BEC CT2: Bed & Care-Place Management (p. 569)	Proposed Qualified care-place state and local-capacity aggregation inform operational scheduling without taking source slot or staffing ownership. Examples: Feature: Bed Availability Tracking; Feature: Bed State Progression; Feature: Bed Readiness Notification; Function: Maintain Operational Bed State; Service: Bed Availability Query; Function: Manage Bed Turnover Progression
BC CT2: Care Scheduling & Capacity Management (p. 31)	BEC CT2: Service Capacity Management (p. 581)	Proposed Qualified care-place state and local-capacity aggregation inform operational scheduling without taking source slot or staffing ownership. Examples: Feature: Operational Capacity Metric Aggregation; Function: Aggregate Service Capacity Telemetry; Service: Regional Health System Capacity Query
BC CT2: Care Coordination & Transition (p. 37)	BEC CT2: Discharge Management (p. 645)	Proposed Operational readiness, shared plan context and discussion support transition while clinical clearance and responsibility transfer retain their source authority. Examples: Feature: Discharge Milestone Tracking; Feature: Discharge Readiness & Coordination Oversight; Feature: Discharge Notification Dispatch; Function: Manage Discharge Coordination Progression; Service: Discharge Readiness Telemetry

From	To	Assertion / rationale
BC CT2: Care Coordination & Transition (p. 37)	BEC CT2: Clinical Collaboration (p. 756)	Proposed Operational readiness, shared plan context and discussion support transition while clinical clearance and responsibility transfer retain their source authority. Examples: Feature: Care-Team Clinical Collaboration Coordination; Feature: In-Conversation LHR Query Resolution; Feature: Zero-PHI Collaboration Metadata Governance; Function: Coordinate Care-Team Collaboration; Service: Clinical Collaboration Space Service; Function: Resolve LHR Query within Collaboration
BC CT2: Care Coordination & Transition (p. 37)	BEC CT2: Care Plan Administration (p. 455)	Proposed Operational readiness, shared plan context and discussion support transition while clinical clearance and responsibility transfer retain their source authority. Examples: Feature: Care Plan Ingestion & Distribution; Function: Receive Care Plan; Service: Care Plan Ingress; Function: Associate Care Plan Context; Service: Care Plan Context Query; Function: Distribute Care Plan Change
BC CT2: Care Support Logistics Management (p. 41)	BEC CT2: Patient Transport (p. 632)	Proposed Physical movement and support work need operational requests, custody/progression evidence and eligible dispatch; clinical work remains externally governed. Examples: Feature: Transport Request Ingestion; Feature: Transport Dispatch & Progress Tracking; Function: Manage Patient Transport Progression; Service: Patient Transport Tracking Service
BC CT2: Care Support Logistics Management (p. 41)	BEC CT2: Clinical Logistics Coordination (p. 639)	Proposed Physical movement and support work need operational requests, custody/progression evidence and eligible dispatch; clinical work remains externally governed. Examples: Feature: Pathology Specimen Transport Tracking; Function: Manage Specimen Transport Progression; Service: Specimen Transit Tracking Service
BC CT2: Care Support Logistics Management (p. 41)	BEC CT2: Work Allocation & Dispatch (p. 607)	Proposed Physical movement and support work need operational requests, custody/progression evidence and eligible dispatch; clinical work remains externally governed. Examples: Feature: Work Item Instantiation; Feature: Worker Matching & Allocation; Feature: Work Dispatch Delivery; Feature: Work Progress Oversight & Escalation; Function: Dispatch Operational Work Order; Service: Work Dispatch Service
BC CT2: Diagnostic Support Provision (p. 51)	BEC CT2: Diagnostic Administration (p. 399)	Proposed Requisition, report and outcome correlation supports diagnostic provision without interpreting specimens or images. Examples: Feature: Diagnostic Request Correlation; Feature: Diagnostic Report Ingestion & Binding; Feature: Diagnostic Report Distribution; Function: Correlate Diagnostic Request; Service: Diagnostic Correlation Resolution; Function: Bind Diagnostic Report

From	To	Assertion / rationale
BC CT2: Diagnostic Support Provision (p. 51)	BEC CT3: Diagnostic Service Integration (p. 401)	Proposed Requisition, report and outcome correlation supports diagnostic provision without interpreting specimens or images. Examples: Feature: Laboratory & Imaging Result Distribution; Feature: Diagnostic History Consolidation; Function: Receive Diagnostic Requisition Feed; Service: Modality Worklist Distribution; Function: Publish Governed Diagnostic Report; Service: Diagnostic Publication Service
BC CT2: Diagnostic Support Provision (p. 51)	BEC CT2: Order Administration (p. 383)	Proposed Requisition, report and outcome correlation supports diagnostic provision without interpreting specimens or images. Examples: Feature: Order Request Ingestion; Feature: Order Destination Resolution & Routing; Feature: Order Closed-Loop Progression Tracking; Feature: Order Cancellation & Modification Coordination; Function: Receive Order Request; Service: Order Requisition Ingress
BC CT2: Medication & Pharmacy Services (p. 58)	BEC CT2: Medication Administration (p. 417)	Proposed Source prescription, dispensing, administration and allergy/history information supports pharmacy activity without Harmonia prescribing or dispensing. Examples: Feature: Medication Order Ingestion; Feature: Dispense Event Tracking; Feature: Medication Administration Record (MAR) Ingestion; Function: Receive Medication Order; Service: Medication Order Ingress; Function: Track Dispense Event
BC CT2: Medication & Pharmacy Services (p. 58)	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Proposed Source prescription, dispensing, administration and allergy/history information supports pharmacy activity without Harmonia prescribing or dispensing. Examples: Feature: Medication History Reconciliation Ingestion; Feature: Medication Administration Tracking; Function: Project Longitudinal Medication History; Service: Longitudinal Medication History Query; Function: Correlate Adverse Reaction & Allergy Profile; Service: Allergy & Adverse Reaction Query
BC CT2: Clinical Technology Management (p. 66)	BEC CT2: Clinical Device Administration (p. 305)	Proposed Device identity, associations and operational equipment context support technology use; maintenance, regulatory and lifecycle decisions retain their owners. Examples: Function: Maintain Device Definition; Service: Device Definition Query; Function: Maintain Device Instance Identity; Service: Device Instance Resolution; Function: Maintain Device Association; Service: Device Association Query

From	To	Assertion / rationale
BC CT2: Clinical Technology Management (p. 66)	BEC CT3: Clinical Resource Management (p. 889)	Historical assertion / Proposed Device identity, associations and operational equipment context support technology use; maintenance, regulatory and lifecycle decisions retain their owners. Examples: Feature: Mobile Clinical Equipment Tracking; Function: Track Mobile Clinical Asset Operational State; Service: Clinical Resource Availability Query
BC CT2: Health Information Management (p. 105)	BEC CT2: Patient Clinical Record (p. 656)	Proposed Document lifecycle, longitudinal synthesis and payload association support governed information without collapsing legal originals, projections and custody. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BC CT2: Health Information Management (p. 105)	BEC CT2: Clinical Record Administration (p. 443)	Proposed Document lifecycle, longitudinal synthesis and payload association support governed information without collapsing legal originals, projections and custody. Examples: Feature: Clinical Document Ingestion; Feature: Document Versioning & Supersession; Feature: Document Metadata Indexing; Function: Receive Clinical Document; Service: Clinical Document Ingress; Function: Govern Clinical Document Lifecycle
BC CT2: Health Information Management (p. 105)	BEC CT3: Linked Information Curation (p. 663)	Proposed Document lifecycle, longitudinal synthesis and payload association support governed information without collapsing legal originals, projections and custody. Examples: Feature: Binary Content Resolution & Assembly; Function: Resolve Associated Binary Content; Function: Assemble Associated Binary Content; Service: Associated Binary Content Retrieval
BC CT2: Clinical Knowledge Management (p. 112)	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed Governed terminology and approved reference content support knowledge use while external ontology authority remains explicit. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service
BC CT2: Clinical Knowledge Management (p. 112)	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed Governed terminology and approved reference content support knowledge use while external ontology authority remains explicit. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query

From	To	Assertion / rationale
BC CT2: Health Information Standards Governance (p. 121)	BEC CT2: Information Design Governance (p. 792)	Proposed Model, dictionary, profile and conformance responsibilities make approved standards operationally usable without replacing standards authority. Examples: Feature: Information Standards & Semantic Governance; Feature: Semantic Conformance Verification; Function: Govern Information Semantics; Service: Semantic Model Definition Service; Function: Govern Information Definition; Service: Information Dictionary Query
BC CT2: Clinical Semantics Governance (p. 128)	BEC CT2: Information Design Governance (p. 792)	Proposed Definition bindings and terminology resolution support shared meaning; runtime coding and ontology approval remain distinct. Examples: Feature: Information Standards & Semantic Governance; Feature: Semantic Conformance Verification; Function: Govern Information Semantics; Service: Semantic Model Definition Service; Function: Govern Information Definition; Service: Information Dictionary Query
BC CT2: Clinical Semantics Governance (p. 128)	BEC CT2: Clinical Knowledge Services (p. 742)	Proposed Definition bindings and terminology resolution support shared meaning; runtime coding and ontology approval remain distinct. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service
BC CT2: Health Information Exchange (p. 136)	BEC CT2: Health Information Exchange Enablement (p. 686)	Proposed Harmonia coordinates bounded information exchange under standards contracts; organisational sharing agreements and domain content retain their authority. Examples: Feature: Direct Submission Ingestion & Delivery; Feature: Clinical Record Retrieval Mediation; Feature: Addressed Clinical Distribution; Feature: Event-Driven Syndication; Function: Receive Information Submission; Service: HIE Submission Gateway Service
BC CT2: Health Information Exchange (p. 136)	BEC CT3: Health Information Communication (p. 890)	Historical assertion / Proposed Harmonia coordinates bounded information exchange under standards contracts; organisational sharing agreements and domain content retain their authority. Examples: Feature: Standards-Based Boundary Exchange; Function: Mediate Standards-Based Health Information Communication; Service: Standards Communication Gateway Service

From	To	Assertion / rationale
BC CT2: Digital Communication Channel (p. 138)	BEC CT2: Clinical Communication Administration (p. 476)	Proposed Directed clinical message handling and operational dialogue support different communication purposes without making a conversation a clinical record. Examples: Feature: Clinical Communication Distribution; Feature: Delivery Acknowledgement Tracking; Feature: Communication Audit Logging; Function: Distribute Clinical Communication; Service: Secure Clinical Message Dispatch; Function: Track Business Delivery Acknowledgement
BC CT2: Digital Communication Channel (p. 138)	BEC CT2: General Collaboration (p. 768)	Proposed Directed clinical message handling and operational dialogue support different communication purposes without making a conversation a clinical record. Examples: Feature: Operational Team Chat Enablement; Function: Coordinate Operational Collaboration; Service: Operational Collaboration Service
BC CT2: Digital Identity & Access Governance (p. 141)	BEC CT2: Health Information Control (p. 715)	Proposed Trusted identity intake, effective policy and patient restrictions support governed access without owning external authentication or consent authority. Examples: Feature: Health Information Access & Consent Control; Feature: Security Context Propagation; Feature: Non-PHI Compliance Auditing; Function: Evaluate Information Access Authority; Service: Policy Evaluation & Authorisation Service; Function: Evaluate Consent Constraint
BC CT2: Digital Identity & Access Governance (p. 141)	BEC CT3: Client Privacy Guardrail Management (p. 237)	Proposed Trusted identity intake, effective policy and patient restrictions support governed access without owning external authentication or consent authority. Examples: Feature: Consent Directive Evaluation; Feature: Confidentiality Flag Enforcement; Function: Evaluate Client Consent; Service: Consent Decision Service; Function: Evaluate Client Confidentiality Restrictions; Service: Confidentiality Restriction Evaluation
BC CT2: Digital Accountability & Trust (p. 155)	BEC CT3: Significant Activity Audit Capture (p. 961)	Historical assertion / Proposed Attribution, activity evidence and source standing answer different accountability questions; evidence alone is not an assurance conclusion. Examples: Feature: Mandatory Significant Activity Audit Capture; Function: Capture Required Significant Audit Evidence; Service: Mandatory Audit Evidence Capture
BC CT2: Digital Accountability & Trust (p. 155)	BEC CT3: Data Provenance Capture (p. 733)	Proposed Attribution, activity evidence and source standing answer different accountability questions; evidence alone is not an assurance conclusion. Examples: Feature: Information Transformation Lineage; Function: Record Significant Information Lineage

From	To	Assertion / rationale
BC CT2: Digital Accountability & Trust (p. 155)	BEC CT3: Information Authority Representation (p. 959)	Historical assertion / Proposed Attribution, activity evidence and source standing answer different accountability questions; evidence alone is not an assurance conclusion. Examples: Feature: Information Authority & Source Standing Representation; Function: Qualify Information Authority Standing
BC CT2: Research Information Support (p. 160)	BEC CT2: Research Information Curation (p. 812)	Proposed Provides existing information under explicitly authorised research-use constraints without taking research governance or trial management. Examples: Feature: Authorised Research Release Assembly; Function: Assemble Purpose-Qualified Research Release; Service: Authorised Research Information Export
BC CT2: Population Health Surveillance (p. 94)	BEC CT2: Population Health Information Reporting (p. 817)	Proposed Source information and required notification progression support surveillance without originating population-health conclusions or statutory duties. Examples: Feature: Population Notification Obligation Tracking
BC CT2: Population Health Surveillance (p. 94)	BEC CT2: Health Information Access (p. 701)	Proposed Source information and required notification progression support surveillance without originating population-health conclusions or statutory duties. Examples: Feature: Federated Clinical Query; Feature: Access-Controlled Information Filtering; Function: Search Health Information; Service: Clinical Information Search Service; Function: Retrieve Health Information; Service: Clinical Resource Retrieval Service
BC CT2: Health-System Planning (p. 99)	BEC CT2: Service Planning Data Management (p. 818)	Proposed Qualified observed demand and capacity evidence supports planning without determining commissioned capacity or service distribution. Examples: Feature: Demand & Capacity Planning Projection
BC CT2: Health Workforce Management (p. 177)	BEC CT2: Provider Administration (p. 243)	Proposed Professional standing, qualified checks and source coverage information support workforce activity without managing employment or granting clinical privilege. Examples: Function: Verify Practitioner Registration; Service: Practitioner Verification Service; Function: Govern Practitioner Profile; Service: Practitioner Directory Query; Function: Maintain Practitioner Roles; Service: Practitioner Role Resolution
BC CT2: Health Workforce Management (p. 177)	BEC CT2: Clinical Qualification Administration (p. 621)	Proposed Professional standing, qualified checks and source coverage information support workforce activity without managing employment or granting clinical privilege. Examples: Feature: Operational Credential Check; Function: Verify Clinical Qualification for Work Dispatch

From	To	Assertion / rationale
BC CT2: Health Workforce Management (p. 177)	BEC CT2: Clinical Rostering (p. 589)	Proposed Professional standing, qualified checks and source coverage information support workforce activity without managing employment or granting clinical privilege.
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	BEC CT2: Organisation Administration (p. 263)	Proposed Represented entity identities and service mappings support structure decisions; mapped entity owners retain authority. Examples: Function: Verify Healthcare Organisation; Service: Organisation Verification Service; Function: Govern Organisation Profile; Service: Organisation Directory Query; Function: Maintain Organisational Structure; Service: Organisational Hierarchy Resolution
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	BEC CT2: Location Administration (p. 276)	Proposed Represented entity identities and service mappings support structure decisions; mapped entity owners retain authority. Examples: Function: Maintain Healthcare Location; Service: Location Directory Query; Function: Maintain Physical Location Hierarchy; Service: Location Hierarchy Resolution; Function: Maintain Care-Place Definition; Service: Care-Place Specification Lookup
BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	BEC CT2: Health Service Administration (p. 287)	Proposed Represented entity identities and service mappings support structure decisions; mapped entity owners retain authority. Examples: Function: Maintain Healthcare Service Definition; Service: Healthcare Service Directory Query; Function: Maintain Service / Location / Provider Map; Service: Service Provision Resolution; Function: Maintain Service Availability; Service: Service Availability Query
BC CT2: Financial & Commercial Services (p. 186)	BEC CT2: Funding & Coverage Administration (p. 487)	Proposed Coverage flags and activity/charging exports support responsible financial systems without financial governance or claims adjudication.
BC CT2: Financial & Commercial Services (p. 186)	BEC CT2: Clinical Charging & Claims Administration (p. 489)	Proposed Coverage flags and activity/charging exports support responsible financial systems without financial governance or claims adjudication.
BC CT1: Health Service Governance (p. 195)	BEC CT3: Governance Requirement Publication Support (p. 962)	Historical assertion / Proposed Proposed bounded publication support makes the governing requirement available to consumers without granting the platform requirement approval authority. Examples: Feature: Governance Requirement Version Publication
BC CT3: Health Service Operational Readiness (p. 46)	BEC CT3: Service Offering Readiness Coordination (p. 966)	Historical assertion / Proposed Proposed evidence-based coordination supports collective deliverability while dependency owners retain control. Examples: Feature: Service Readiness Dependency Assessment; Function: Assess Service Offering Readiness; Service: Service Offering Readiness Query

From	To	Assertion / rationale
BC CT2: Clinical Technology Management (p. 66)	BEC CT3: Device Utilisation Administration (p. 311)	Proposed Reference repair following explicit responsibility consolidation/decomposition; original assertion and its standing remain in the history.

32.8. Governed Process

From	To	Assertion / rationale
Function: Apply Governed Person Identity Correction (p. 222)	Process: Governed Person Identity Correction Process (p. 224)	Established em:35 (p. 1435)
Function: Verify Practitioner Registration (p. 254)	Process: Practitioner Verification Process (p. 262)	Established em:94 (p. 1437)
Function: Manage Referral Progression (p. 352)	Process: Referral Progression Process (p. 354)	Established sa:41 (p. 1441)
Function: Progress Encounter State (p. 378)	Process: Encounter Lifecycle Process (p. 381)	Established sa:73 (p. 1442)
Function: Manage Order Progression (p. 393)	Process: Closed-Loop Order Progression Process (p. 398)	Established sa:97 (p. 1443)
Function: Govern Clinical Document Lifecycle (p. 451)	Process: Clinical Document Lifecycle Process (p. 454)	Established sa:173 (p. 1445)
Function: Manage Clinic Queue Progression (p. 521)	Process: Clinic / Operational Queue Progression Process (p. 523)	Established hso:84 (p. 1457)
Function: Manage Theatre Case Progression (p. 545)	Process: Theatre Case Progression Process (p. 547)	Established hso:115 (p. 1458)
Function: Manage Bed Turnover Progression (p. 578)	Process: Bed Turnover Process (p. 580)	Established hso:163 (p. 1459)
Function: Manage Operational Work Progression (p. 615)	Process: Operational Work Progression Process (p. 617)	Established hso:233 (p. 1462)
Function: Manage Patient Transport Progression (p. 637)	Process: Patient Transport Process (p. 638)	Established hso:255 (p. 1462)
Function: Manage Specimen Transport Progression (p. 902)	Process: Specimen Transport Process (p. 916)	Historical assertion / Established hso:269 (p. 1463)
Function: Manage Discharge Coordination Progression (p. 651)	Process: Discharge Progression Process (p. 652)	Established hso:284 (p. 1463)
Function: Coordinate Work Order (p. 904)	Process: Work Order Progression Process (p. 917)	Historical assertion / Established ise:183 (p. 1469)
Function: Coordinate To Do (p. 905)	Process: To Do Progression Process (p. 918)	Historical assertion / Established ise:187 (p. 1469)
Function: Coordinate Synthetic Task (p. 907)	Process: Synthetic Task Progression Process (p. 919)	Historical assertion / Established ise:191 (p. 1469)

32.9. Uses Process

From	To	Assertion / rationale
Service: Request Service Assurance (p. 913)	Process: Assurance Process Execution (p. 920)	Historical assertion / Established assurance:170 (p. 1430)

32.10. Exposes Process

From	To	Assertion / rationale
Service: Communicate Service Assurance Outcome (p. 915)	Process: Assurance Process Reporting / Communication (p. 922)	Historical assertion / Established assurance:195 (p. 1430)

32.11. Uses Function

From	To	Assertion / rationale
Service: Associated Binary Content Retrieval (p. 668)	Function: Resolve Associated Binary Content (p. 666)	Proposed The exposed client contract is fulfilled using the bounded state-changing behaviour; no one-to-one generation rule is implied.
Service: Associated Binary Content Retrieval (p. 668)	Function: Assemble Associated Binary Content (p. 667)	Proposed The exposed client contract is fulfilled using the bounded state-changing behaviour; no one-to-one generation rule is implied.
Service: Mandatory Audit Evidence Capture (p. 985)	Function: Capture Required Significant Audit Evidence (p. 984)	Historical assertion / Proposed The exposed client contract is fulfilled using the bounded state-changing behaviour; no one-to-one generation rule is implied.
Service: Entity Operational Input Submission (p. 987)	Function: Coordinate Entity-Relative Input (p. 986)	Historical assertion / Proposed The exposed client contract is fulfilled using the bounded state-changing behaviour; no one-to-one generation rule is implied.
Service: Service Offering Readiness Query (p. 995)	Function: Assess Service Offering Readiness (p. 994)	Historical assertion / Proposed The exposed client contract is fulfilled using the bounded state-changing behaviour; no one-to-one generation rule is implied.
Service: Authorised Research Information Export (p. 815)	Function: Assemble Purpose-Qualified Research Release (p. 814)	Proposed The exposed client contract is fulfilled using the bounded state-changing behaviour; no one-to-one generation rule is implied.

32.12. Enterprise Enabled By

From	To	Assertion / rationale
BEC CT3: Person Identity Administration (p. 214)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Identifier Resolution; Feature: Cross-Authority Identifier Correlation; Feature: Identity Alias Association; Feature: Governed Identity Correction; Function: Resolve Person Identifier; Service: Person Identifier Resolution
BEC CT3: Person Identity Administration (p. 214)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Identifier Resolution; Feature: Cross-Authority Identifier Correlation; Feature: Identity Alias Association; Feature: Governed Identity Correction; Function: Resolve Person Identifier; Service: Person Identifier Resolution
BEC CT3: Person Identity Administration (p. 214)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Identifier Resolution; Feature: Cross-Authority Identifier Correlation; Feature: Identity Alias Association; Feature: Governed Identity Correction; Function: Resolve Person Identifier; Service: Person Identifier Resolution
BEC CT3: Person Identity Administration (p. 214)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Identifier Resolution; Feature: Cross-Authority Identifier Correlation; Feature: Identity Alias Association; Feature: Governed Identity Correction; Function: Resolve Person Identifier; Service: Person Identifier Resolution
BEC CT3: Healthcare Subject Profile Administration (p. 226)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Subject Context Binding; Feature: Demographic Context Governance; Function: Establish Healthcare Subject Context; Service: Healthcare Subject Context Resolution; Function: Govern Subject Demographic Context; Service: Demographic Change Distribution

From	To	Assertion / rationale
BEC CT3: Healthcare Subject Profile Administration (p. 226)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Subject Context Binding; Feature: Demographic Context Governance; Function: Establish Healthcare Subject Context; Service: Healthcare Subject Context Resolution; Function: Govern Subject Demographic Context; Service: Demographic Change Distribution
BEC CT3: Healthcare Subject Profile Administration (p. 226)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Subject Context Binding; Feature: Demographic Context Governance; Function: Establish Healthcare Subject Context; Service: Healthcare Subject Context Resolution; Function: Govern Subject Demographic Context; Service: Demographic Change Distribution
BEC CT3: Healthcare Subject Profile Administration (p. 226)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Feature: Subject Context Binding; Feature: Demographic Context Governance; Function: Establish Healthcare Subject Context; Service: Healthcare Subject Context Resolution; Function: Govern Subject Demographic Context; Service: Demographic Change Distribution
BEC CT2: Provider Administration (p. 243)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Practitioner Registration; Service: Practitioner Verification Service; Function: Govern Practitioner Profile; Service: Practitioner Directory Query; Function: Maintain Practitioner Roles; Service: Practitioner Role Resolution
BEC CT2: Provider Administration (p. 243)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Practitioner Registration; Service: Practitioner Verification Service; Function: Govern Practitioner Profile; Service: Practitioner Directory Query; Function: Maintain Practitioner Roles; Service: Practitioner Role Resolution

From	To	Assertion / rationale
BEC CT2: Provider Administration (p. 243)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Practitioner Registration; Service: Practitioner Verification Service; Function: Govern Practitioner Profile; Service: Practitioner Directory Query; Function: Maintain Practitioner Roles; Service: Practitioner Role Resolution
BEC CT2: Provider Administration (p. 243)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Practitioner Registration; Service: Practitioner Verification Service; Function: Govern Practitioner Profile; Service: Practitioner Directory Query; Function: Maintain Practitioner Roles; Service: Practitioner Role Resolution
BEC CT2: Organisation Administration (p. 263)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Healthcare Organisation; Service: Organisation Verification Service; Function: Govern Organisation Profile; Service: Organisation Directory Query; Function: Maintain Organisational Structure; Service: Organisational Hierarchy Resolution
BEC CT2: Organisation Administration (p. 263)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Healthcare Organisation; Service: Organisation Verification Service; Function: Govern Organisation Profile; Service: Organisation Directory Query; Function: Maintain Organisational Structure; Service: Organisational Hierarchy Resolution
BEC CT2: Organisation Administration (p. 263)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Healthcare Organisation; Service: Organisation Verification Service; Function: Govern Organisation Profile; Service: Organisation Directory Query; Function: Maintain Organisational Structure; Service: Organisational Hierarchy Resolution

From	To	Assertion / rationale
BEC CT2: Organisation Administration (p. 263)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Verify Healthcare Organisation; Service: Organisation Verification Service; Function: Govern Organisation Profile; Service: Organisation Directory Query; Function: Maintain Organisational Structure; Service: Organisational Hierarchy Resolution
BEC CT2: Location Administration (p. 276)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Location; Service: Location Directory Query; Function: Maintain Physical Location Hierarchy; Service: Location Hierarchy Resolution; Function: Maintain Care-Place Definition; Service: Care-Place Specification Lookup
BEC CT2: Location Administration (p. 276)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Location; Service: Location Directory Query; Function: Maintain Physical Location Hierarchy; Service: Location Hierarchy Resolution; Function: Maintain Care-Place Definition; Service: Care-Place Specification Lookup
BEC CT2: Location Administration (p. 276)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Location; Service: Location Directory Query; Function: Maintain Physical Location Hierarchy; Service: Location Hierarchy Resolution; Function: Maintain Care-Place Definition; Service: Care-Place Specification Lookup
BEC CT2: Location Administration (p. 276)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Location; Service: Location Directory Query; Function: Maintain Physical Location Hierarchy; Service: Location Hierarchy Resolution; Function: Maintain Care-Place Definition; Service: Care-Place Specification Lookup

From	To	Assertion / rationale
BEC CT2: Health Service Administration (p. 287)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Service Definition; Service: Healthcare Service Directory Query; Function: Maintain Service / Location / Provider Map; Service: Service Provision Resolution; Function: Maintain Service Availability; Service: Service Availability Query
BEC CT2: Health Service Administration (p. 287)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Service Definition; Service: Healthcare Service Directory Query; Function: Maintain Service / Location / Provider Map; Service: Service Provision Resolution; Function: Maintain Service Availability; Service: Service Availability Query
BEC CT2: Health Service Administration (p. 287)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Service Definition; Service: Healthcare Service Directory Query; Function: Maintain Service / Location / Provider Map; Service: Service Provision Resolution; Function: Maintain Service Availability; Service: Service Availability Query
BEC CT2: Health Service Administration (p. 287)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Healthcare Service Definition; Service: Healthcare Service Directory Query; Function: Maintain Service / Location / Provider Map; Service: Service Provision Resolution; Function: Maintain Service Availability; Service: Service Availability Query
BEC CT2: Clinical Device Administration (p. 305)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Device Definition; Service: Device Definition Query; Function: Maintain Device Instance Identity; Service: Device Instance Resolution; Function: Maintain Device Association; Service: Device Association Query

From	To	Assertion / rationale
BEC CT2: Clinical Device Administration (p. 305)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Device Definition; Service: Device Definition Query; Function: Maintain Device Instance Identity; Service: Device Instance Resolution; Function: Maintain Device Association; Service: Device Association Query
BEC CT2: Clinical Device Administration (p. 305)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Device Definition; Service: Device Definition Query; Function: Maintain Device Instance Identity; Service: Device Instance Resolution; Function: Maintain Device Association; Service: Device Association Query
BEC CT2: Clinical Device Administration (p. 305)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Entity/profile resolution, qualified relationships and non-destructive lifecycle recur across persons, providers, organisations, places, services and devices. Examples: Function: Maintain Device Definition; Service: Device Definition Query; Function: Maintain Device Instance Identity; Service: Device Instance Resolution; Function: Maintain Device Association; Service: Device Association Query
BEC CT3: Client Privacy Guardrail Management (p. 237)	Enterprise CT2: Policy & Control (p. 865)	Proposed Consent-qualified use and governed action need effective policy, trusted context and attributable significant decisions. Examples: Feature: Consent Directive Evaluation; Feature: Confidentiality Flag Enforcement; Function: Evaluate Client Consent; Service: Consent Decision Service; Function: Evaluate Client Confidentiality Restrictions; Service: Confidentiality Restriction Evaluation
BEC CT3: Client Privacy Guardrail Management (p. 237)	Enterprise CT2: Context Management (p. 839)	Proposed Consent-qualified use and governed action need effective policy, trusted context and attributable significant decisions. Examples: Feature: Consent Directive Evaluation; Feature: Confidentiality Flag Enforcement; Function: Evaluate Client Consent; Service: Consent Decision Service; Function: Evaluate Client Confidentiality Restrictions; Service: Confidentiality Restriction Evaluation
BEC CT3: Client Privacy Guardrail Management (p. 237)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Consent-qualified use and governed action need effective policy, trusted context and attributable significant decisions. Examples: Feature: Consent Directive Evaluation; Feature: Confidentiality Flag Enforcement; Function: Evaluate Client Consent; Service: Consent Decision Service; Function: Evaluate Client Confidentiality Restrictions; Service: Confidentiality Restriction Evaluation

From	To	Assertion / rationale
BEC CT2: Health Information Control (p. 715)	Enterprise CT2: Policy & Control (p. 865)	Proposed Consent-qualified use and governed action need effective policy, trusted context and attributable significant decisions. Examples: Feature: Health Information Access & Consent Control; Feature: Security Context Propagation; Feature: Non-PHI Compliance Auditing; Function: Evaluate Information Access Authority; Service: Policy Evaluation & Authorisation Service; Function: Evaluate Consent Constraint
BEC CT2: Health Information Control (p. 715)	Enterprise CT2: Context Management (p. 839)	Proposed Consent-qualified use and governed action need effective policy, trusted context and attributable significant decisions. Examples: Feature: Health Information Access & Consent Control; Feature: Security Context Propagation; Feature: Non-PHI Compliance Auditing; Function: Evaluate Information Access Authority; Service: Policy Evaluation & Authorisation Service; Function: Evaluate Consent Constraint
BEC CT2: Health Information Control (p. 715)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Consent-qualified use and governed action need effective policy, trusted context and attributable significant decisions. Examples: Feature: Health Information Access & Consent Control; Feature: Security Context Propagation; Feature: Non-PHI Compliance Auditing; Function: Evaluate Information Access Authority; Service: Policy Evaluation & Authorisation Service; Function: Evaluate Consent Constraint
BEC CT2: Referral Administration (p. 342)	Enterprise CT2: Context Management (p. 839)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Referral Document Ingestion; Feature: Referral Supporting Information; Feature: Referral Status & Outcome Tracking; Function: Receive Referral; Service: Referral Submission Service; Function: Assemble Referral Context
BEC CT2: Referral Administration (p. 342)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Referral Document Ingestion; Feature: Referral Supporting Information; Feature: Referral Status & Outcome Tracking; Function: Receive Referral; Service: Referral Submission Service; Function: Assemble Referral Context

From	To	Assertion / rationale
BEC CT2: Referral Administration (p. 342)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Referral Document Ingestion; Feature: Referral Supporting Information; Feature: Referral Status & Outcome Tracking; Function: Receive Referral; Service: Referral Submission Service; Function: Assemble Referral Context
BEC CT2: Referral Administration (p. 342)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Referral Document Ingestion; Feature: Referral Supporting Information; Feature: Referral Status & Outcome Tracking; Function: Receive Referral; Service: Referral Submission Service; Function: Assemble Referral Context
BEC CT2: Order Administration (p. 383)	Enterprise CT2: Context Management (p. 839)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Order Request Ingestion; Feature: Order Destination Resolution & Routing; Feature: Order Closed-Loop Progression Tracking; Feature: Order Cancellation & Modification Coordination; Function: Receive Order Request; Service: Order Requisition Ingress
BEC CT2: Order Administration (p. 383)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Order Request Ingestion; Feature: Order Destination Resolution & Routing; Feature: Order Closed-Loop Progression Tracking; Feature: Order Cancellation & Modification Coordination; Function: Receive Order Request; Service: Order Requisition Ingress
BEC CT2: Order Administration (p. 383)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Order Request Ingestion; Feature: Order Destination Resolution & Routing; Feature: Order Closed-Loop Progression Tracking; Feature: Order Cancellation & Modification Coordination; Function: Receive Order Request; Service: Order Requisition Ingress

From	To	Assertion / rationale
BEC CT2: Order Administration (p. 383)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Order Request Ingestion; Feature: Order Destination Resolution & Routing; Feature: Order Closed-Loop Progression Tracking; Feature: Order Cancellation & Modification Coordination; Function: Receive Order Request; Service: Order Requisition Ingress
BEC CT2: Diagnostic Administration (p. 399)	Enterprise CT2: Context Management (p. 839)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Diagnostic Request Correlation; Feature: Diagnostic Report Ingestion & Binding; Feature: Diagnostic Report Distribution; Function: Correlate Diagnostic Request; Service: Diagnostic Correlation Resolution; Function: Bind Diagnostic Report
BEC CT2: Diagnostic Administration (p. 399)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Diagnostic Request Correlation; Feature: Diagnostic Report Ingestion & Binding; Feature: Diagnostic Report Distribution; Function: Correlate Diagnostic Request; Service: Diagnostic Correlation Resolution; Function: Bind Diagnostic Report
BEC CT2: Diagnostic Administration (p. 399)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Diagnostic Request Correlation; Feature: Diagnostic Report Ingestion & Binding; Feature: Diagnostic Report Distribution; Function: Correlate Diagnostic Request; Service: Diagnostic Correlation Resolution; Function: Bind Diagnostic Report
BEC CT2: Diagnostic Administration (p. 399)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Diagnostic Request Correlation; Feature: Diagnostic Report Ingestion & Binding; Feature: Diagnostic Report Distribution; Function: Correlate Diagnostic Request; Service: Diagnostic Correlation Resolution; Function: Bind Diagnostic Report

From	To	Assertion / rationale
BEC CT2: Medication Administration (p. 417)	Enterprise CT2: Context Management (p. 839)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Medication Order Ingestion; Feature: Dispense Event Tracking; Feature: Medication Administration Record (MAR) Ingestion; Function: Receive Medication Order; Service: Medication Order Ingress; Function: Track Dispense Event
BEC CT2: Medication Administration (p. 417)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Medication Order Ingestion; Feature: Dispense Event Tracking; Feature: Medication Administration Record (MAR) Ingestion; Function: Receive Medication Order; Service: Medication Order Ingress; Function: Track Dispense Event
BEC CT2: Medication Administration (p. 417)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Medication Order Ingestion; Feature: Dispense Event Tracking; Feature: Medication Administration Record (MAR) Ingestion; Function: Receive Medication Order; Service: Medication Order Ingress; Function: Track Dispense Event
BEC CT2: Medication Administration (p. 417)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Medication Order Ingestion; Feature: Dispense Event Tracking; Feature: Medication Administration Record (MAR) Ingestion; Function: Receive Medication Order; Service: Medication Order Ingress; Function: Track Dispense Event
BEC CT2: Procedure Administration (p. 435)	Enterprise CT2: Context Management (p. 839)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Procedure Booking Ingestion; Feature: Procedural Documentation Ingestion; Function: Receive Procedure Booking / Request; Service: Procedure Booking Ingress; Function: Receive Procedural Documentation; Service: Procedural Documentation Ingress

From	To	Assertion / rationale
BEC CT2: Procedure Administration (p. 435)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Procedure Booking Ingestion; Feature: Procedural Documentation Ingestion; Function: Receive Procedure Booking / Request; Service: Procedure Booking Ingress; Function: Receive Procedural Documentation; Service: Procedural Documentation Ingress
BEC CT2: Procedure Administration (p. 435)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Procedure Booking Ingestion; Feature: Procedural Documentation Ingestion; Function: Receive Procedure Booking / Request; Service: Procedure Booking Ingress; Function: Receive Procedural Documentation; Service: Procedural Documentation Ingress
BEC CT2: Procedure Administration (p. 435)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Procedure Booking Ingestion; Feature: Procedural Documentation Ingestion; Function: Receive Procedure Booking / Request; Service: Procedure Booking Ingress; Function: Receive Procedural Documentation; Service: Procedural Documentation Ingress
BEC CT2: Episode & Encounter Administration (p. 363)	Enterprise CT2: Context Management (p. 839)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Encounter Context Creation; Feature: Encounter State Progression; Feature: Encounter Bed/Location Association; Function: Establish Encounter Context; Service: Encounter Context Resolution; Function: Progress Encounter State
BEC CT2: Episode & Encounter Administration (p. 363)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Encounter Context Creation; Feature: Encounter State Progression; Feature: Encounter Bed/Location Association; Function: Establish Encounter Context; Service: Encounter Context Resolution; Function: Progress Encounter State

From	To	Assertion / rationale
BEC CT2: Episode & Encounter Administration (p. 363)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Encounter Context Creation; Feature: Encounter State Progression; Feature: Encounter Bed/Location Association; Function: Establish Encounter Context; Service: Encounter Context Resolution; Function: Progress Encounter State
BEC CT2: Episode & Encounter Administration (p. 363)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Transactions bind clinical/service context, preserve distinct lifecycle and acknowledgements, exchange source information and coordinate assigned operational consequences. Examples: Feature: Encounter Context Creation; Feature: Encounter State Progression; Feature: Encounter Bed/Location Association; Function: Establish Encounter Context; Service: Encounter Context Resolution; Function: Progress Encounter State
BEC CT2: Clinical Record Administration (p. 443)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Clinical Document Ingestion; Feature: Document Versioning & Supersession; Feature: Document Metadata Indexing; Function: Receive Clinical Document; Service: Clinical Document Ingress; Function: Govern Clinical Document Lifecycle
BEC CT2: Clinical Record Administration (p. 443)	Enterprise CT2: Information Management (p. 844)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Clinical Document Ingestion; Feature: Document Versioning & Supersession; Feature: Document Metadata Indexing; Function: Receive Clinical Document; Service: Clinical Document Ingress; Function: Govern Clinical Document Lifecycle
BEC CT2: Clinical Record Administration (p. 443)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Clinical Document Ingestion; Feature: Document Versioning & Supersession; Feature: Document Metadata Indexing; Function: Receive Clinical Document; Service: Clinical Document Ingress; Function: Govern Clinical Document Lifecycle

From	To	Assertion / rationale
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT2: Information Management (p. 844)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BEC CT2: Care Plan Administration (p. 455)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Care Plan Ingestion & Distribution; Function: Receive Care Plan; Service: Care Plan Ingress; Function: Associate Care Plan Context; Service: Care Plan Context Query; Function: Distribute Care Plan Change
BEC CT2: Care Plan Administration (p. 455)	Enterprise CT2: Information Management (p. 844)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Care Plan Ingestion & Distribution; Function: Receive Care Plan; Service: Care Plan Ingress; Function: Associate Care Plan Context; Service: Care Plan Context Query; Function: Distribute Care Plan Change

From	To	Assertion / rationale
BEC CT2: Care Plan Administration (p. 455)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Document supersession, longitudinal synthesis and plan revision share managed-state, preservation and attribution needs while keeping different content ownership. Examples: Feature: Care Plan Ingestion & Distribution; Function: Receive Care Plan; Service: Care Plan Ingress; Function: Associate Care Plan Context; Service: Care Plan Context Query; Function: Distribute Care Plan Change
BEC CT2: Health Information Access (p. 701)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Information, terminology and on-call resolution all expose criteria-qualified discovery under policy; the discovered subjects remain different. Examples: Feature: Federated Clinical Query; Feature: Access-Controlled Information Filtering; Function: Search Health Information; Service: Clinical Information Search Service; Function: Retrieve Health Information; Service: Clinical Resource Retrieval Service
BEC CT2: Health Information Access (p. 701)	Enterprise CT2: Policy & Control (p. 865)	Proposed Information, terminology and on-call resolution all expose criteria-qualified discovery under policy; the discovered subjects remain different. Examples: Feature: Federated Clinical Query; Feature: Access-Controlled Information Filtering; Function: Search Health Information; Service: Clinical Information Search Service; Function: Retrieve Health Information; Service: Clinical Resource Retrieval Service
BEC CT2: Clinical Knowledge Services (p. 742)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Information, terminology and on-call resolution all expose criteria-qualified discovery under policy; the discovered subjects remain different. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service
BEC CT2: Clinical Knowledge Services (p. 742)	Enterprise CT2: Policy & Control (p. 865)	Proposed Information, terminology and on-call resolution all expose criteria-qualified discovery under policy; the discovered subjects remain different. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service
BEC CT2: On-Call Management (p. 601)	Enterprise CT2: Search & Discovery (p. 849)	Proposed Information, terminology and on-call resolution all expose criteria-qualified discovery under policy; the discovered subjects remain different. Examples: Feature: Active On-Call Provider Resolution; Function: Resolve Active On-Call Coverage; Service: On-Call Specialist Lookup

From	To	Assertion / rationale
BEC CT2: On-Call Management (p. 601)	Enterprise CT2: Policy & Control (p. 865)	Proposed Information, terminology and on-call resolution all expose criteria-qualified discovery under policy; the discovered subjects remain different. Examples: Feature: Active On-Call Provider Resolution; Function: Resolve Active On-Call Coverage; Service: On-Call Specialist Lookup
BEC CT2: Health Information Exchange Enablement (p. 686)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Direct Submission Ingestion & Delivery; Feature: Clinical Record Retrieval Mediation; Feature: Addressed Clinical Distribution; Feature: Event-Driven Syndication; Function: Receive Information Submission; Service: HIE Submission Gateway Service
BEC CT2: Health Information Exchange Enablement (p. 686)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Direct Submission Ingestion & Delivery; Feature: Clinical Record Retrieval Mediation; Feature: Addressed Clinical Distribution; Feature: Event-Driven Syndication; Function: Receive Information Submission; Service: HIE Submission Gateway Service
BEC CT2: Health Information Exchange Enablement (p. 686)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Direct Submission Ingestion & Delivery; Feature: Clinical Record Retrieval Mediation; Feature: Addressed Clinical Distribution; Feature: Event-Driven Syndication; Function: Receive Information Submission; Service: HIE Submission Gateway Service
BEC CT3: Health Information Communication (p. 890)	Enterprise CT2: Interoperability & Exchange (p. 854)	Historical assertion / Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Standards-Based Boundary Exchange; Function: Mediate Standards-Based Health Information Communication; Service: Standards Communication Gateway Service

From	To	Assertion / rationale
BEC CT3: Health Information Communication (p. 890)	Enterprise CT2: Event & Subscription (p. 856)	Historical assertion / Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Standards-Based Boundary Exchange; Function: Mediate Standards-Based Health Information Communication; Service: Standards Communication Gateway Service
BEC CT3: Health Information Communication (p. 890)	Enterprise CT2: Operational Assurance (p. 873)	Historical assertion / Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Standards-Based Boundary Exchange; Function: Mediate Standards-Based Health Information Communication; Service: Standards Communication Gateway Service
BEC CT2: Clinical Communication Administration (p. 476)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Clinical Communication Distribution; Feature: Delivery Acknowledgement Tracking; Feature: Communication Audit Logging; Function: Distribute Clinical Communication; Service: Secure Clinical Message Dispatch; Function: Track Business Delivery Acknowledgement
BEC CT2: Clinical Communication Administration (p. 476)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Clinical Communication Distribution; Feature: Delivery Acknowledgement Tracking; Feature: Communication Audit Logging; Function: Distribute Clinical Communication; Service: Secure Clinical Message Dispatch; Function: Track Business Delivery Acknowledgement
BEC CT2: Clinical Communication Administration (p. 476)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Clinical Communication Distribution; Feature: Delivery Acknowledgement Tracking; Feature: Communication Audit Logging; Function: Distribute Clinical Communication; Service: Secure Clinical Message Dispatch; Function: Track Business Delivery Acknowledgement

From	To	Assertion / rationale
BEC CT2: Population Health Information Reporting (p. 817)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Population Notification Obligation Tracking
BEC CT2: Population Health Information Reporting (p. 817)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Population Notification Obligation Tracking
BEC CT2: Population Health Information Reporting (p. 817)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Addressed exchange, syndication and notifications require explicit contract/delivery standing and resilient observation rather than equating transport acknowledgement with outcome. Examples: Feature: Population Notification Obligation Tracking
BEC CT3: Primary Care Coordination (p. 669)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: General Practice Shared Record Ingestion; Feature: Primary Care Notification Dispatch; Function: Correlate Primary Care Consultation Context; Service: Primary Care Summary Ingress; Function: Coordinate Multi-Agency Chronic Care Plan; Service: Chronic Care Timeline Query
BEC CT3: Primary Care Coordination (p. 669)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: General Practice Shared Record Ingestion; Feature: Primary Care Notification Dispatch; Function: Correlate Primary Care Consultation Context; Service: Primary Care Summary Ingress; Function: Coordinate Multi-Agency Chronic Care Plan; Service: Chronic Care Timeline Query
BEC CT3: Primary Care Coordination (p. 669)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: General Practice Shared Record Ingestion; Feature: Primary Care Notification Dispatch; Function: Correlate Primary Care Consultation Context; Service: Primary Care Summary Ingress; Function: Coordinate Multi-Agency Chronic Care Plan; Service: Chronic Care Timeline Query

From	To	Assertion / rationale
BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Acute Clinical State Event Capture; Feature: Acute Care Clinical Context Provision; Function: Assemble Acute Clinical Baseline; Service: Acute Admission Dossier Query; Function: Route Acute Critical Result Alert; Service: Acute Clinical Alert Dispatch
BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Acute Clinical State Event Capture; Feature: Acute Care Clinical Context Provision; Function: Assemble Acute Clinical Baseline; Service: Acute Admission Dossier Query; Function: Route Acute Critical Result Alert; Service: Acute Clinical Alert Dispatch
BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Acute Clinical State Event Capture; Feature: Acute Care Clinical Context Provision; Function: Assemble Acute Clinical Baseline; Service: Acute Admission Dossier Query; Function: Route Acute Critical Result Alert; Service: Acute Clinical Alert Dispatch
BEC CT3: Emergency Care Progression (p. 554)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Emergency Department Arrival & Triage Ingestion; Feature: Emergency Care Cross-Facility Correlation; Function: Bind ED Triage Assessment; Service: ED Triage Stream Ingress; Function: Provide Emergency LHR Summary; Service: Emergency Fast-Path LHR Query
BEC CT3: Emergency Care Progression (p. 554)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Emergency Department Arrival & Triage Ingestion; Feature: Emergency Care Cross-Facility Correlation; Function: Bind ED Triage Assessment; Service: ED Triage Stream Ingress; Function: Provide Emergency LHR Summary; Service: Emergency Fast-Path LHR Query

From	To	Assertion / rationale
BEC CT3: Emergency Care Progression (p. 554)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Emergency Department Arrival & Triage Ingestion; Feature: Emergency Care Cross-Facility Correlation; Function: Bind ED Triage Assessment; Service: ED Triage Stream Ingress; Function: Provide Emergency LHR Summary; Service: Emergency Fast-Path LHR Query
BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Inpatient Rounding Context Provision; Feature: Inpatient Clinical Progression Tracking; Function: Consolidate Inpatient Clinical Round Feed; Service: Ward Round Clinical Briefing Query; Function: Assemble Inpatient Discharge Dossier; Service: Discharge Dossier Packaging Service
BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Inpatient Rounding Context Provision; Feature: Inpatient Clinical Progression Tracking; Function: Consolidate Inpatient Clinical Round Feed; Service: Ward Round Clinical Briefing Query; Function: Assemble Inpatient Discharge Dossier; Service: Discharge Dossier Packaging Service
BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Inpatient Rounding Context Provision; Feature: Inpatient Clinical Progression Tracking; Function: Consolidate Inpatient Clinical Round Feed; Service: Ward Round Clinical Briefing Query; Function: Assemble Inpatient Discharge Dossier; Service: Discharge Dossier Packaging Service
BEC CT3: Community Care Coordination (p. 460)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Community Encounter Ingestion; Feature: Community Care Plan Synchronization; Function: Synchronize Community Visit Record; Service: Community Care Encounter Ingress; Function: Bind Community Support Network Context; Service: Community Care Team Query

From	To	Assertion / rationale
BEC CT3: Community Care Coordination (p. 460)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Community Encounter Ingestion; Feature: Community Care Plan Synchronization; Function: Synchronize Community Visit Record; Service: Community Care Encounter Ingress; Function: Bind Community Support Network Context; Service: Community Care Team Query
BEC CT3: Community Care Coordination (p. 460)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Community Encounter Ingestion; Feature: Community Care Plan Synchronization; Function: Synchronize Community Visit Record; Service: Community Care Encounter Ingress; Function: Bind Community Support Network Context; Service: Community Care Team Query
BEC CT3: Outreach Care Continuity (p. 466)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Outreach Visit Context Provision; Feature: Outreach Encounter Outcome Ingestion; Function: Provide Outreach Visit Context; Function: Reconcile Outreach Clinical Batch; Service: Outreach Batch Reconciliation Service
BEC CT3: Outreach Care Continuity (p. 466)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Outreach Visit Context Provision; Feature: Outreach Encounter Outcome Ingestion; Function: Provide Outreach Visit Context; Function: Reconcile Outreach Clinical Batch; Service: Outreach Batch Reconciliation Service
BEC CT3: Outreach Care Continuity (p. 466)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Outreach Visit Context Provision; Feature: Outreach Encounter Outcome Ingestion; Function: Provide Outreach Visit Context; Function: Reconcile Outreach Clinical Batch; Service: Outreach Batch Reconciliation Service

From	To	Assertion / rationale
BEC CT3: Remote Patient Monitoring (p. 675)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Remote Telemetry Ingestion; Feature: Virtual Care Session Context Binding; Function: Correlate Virtual Care Consultation Context; Service: Virtual Encounter Context Resolution; Function: Ingest Remote Patient Telemetry; Service: Remote Telemetry Ingress
BEC CT3: Remote Patient Monitoring (p. 675)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Remote Telemetry Ingestion; Feature: Virtual Care Session Context Binding; Function: Correlate Virtual Care Consultation Context; Service: Virtual Encounter Context Resolution; Function: Ingest Remote Patient Telemetry; Service: Remote Telemetry Ingress
BEC CT3: Remote Patient Monitoring (p. 675)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Remote Telemetry Ingestion; Feature: Virtual Care Session Context Binding; Function: Correlate Virtual Care Consultation Context; Service: Virtual Encounter Context Resolution; Function: Ingest Remote Patient Telemetry; Service: Remote Telemetry Ingress
BEC CT3: Screening and Immunisation Enablement (p. 329)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Immunisation Event Submission; Feature: Screening Recall Notification Distribution; Function: Syndicate Immunisation Record; Service: Immunisation History Query; Function: Project Screening Eligibility Timeline; Service: Preventive Screening Status Query
BEC CT3: Screening and Immunisation Enablement (p. 329)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Immunisation Event Submission; Feature: Screening Recall Notification Distribution; Function: Syndicate Immunisation Record; Service: Immunisation History Query; Function: Project Screening Eligibility Timeline; Service: Preventive Screening Status Query

From	To	Assertion / rationale
BEC CT3: Screening and Immunisation Enablement (p. 329)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Immunisation Event Submission; Feature: Screening Recall Notification Distribution; Function: Syndicate Immunisation Record; Service: Immunisation History Query; Function: Project Screening Eligibility Timeline; Service: Preventive Screening Status Query
BEC CT3: Diagnostic Service Integration (p. 401)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Laboratory & Imaging Result Distribution; Feature: Diagnostic History Consolidation; Function: Receive Diagnostic Requisition Feed; Service: Modality Worklist Distribution; Function: Publish Governed Diagnostic Report; Service: Diagnostic Publication Service
BEC CT3: Diagnostic Service Integration (p. 401)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Laboratory & Imaging Result Distribution; Feature: Diagnostic History Consolidation; Function: Receive Diagnostic Requisition Feed; Service: Modality Worklist Distribution; Function: Publish Governed Diagnostic Report; Service: Diagnostic Publication Service
BEC CT3: Diagnostic Service Integration (p. 401)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Laboratory & Imaging Result Distribution; Feature: Diagnostic History Consolidation; Function: Receive Diagnostic Requisition Feed; Service: Modality Worklist Distribution; Function: Publish Governed Diagnostic Report; Service: Diagnostic Publication Service
BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Enterprise CT2: Context Management (p. 839)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Medication History Reconciliation Ingestion; Feature: Medication Administration Tracking; Function: Project Longitudinal Medication History; Service: Longitudinal Medication History Query; Function: Correlate Adverse Reaction & Allergy Profile; Service: Allergy & Adverse Reaction Query

From	To	Assertion / rationale
BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Enterprise CT2: Information Management (p. 844)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Medication History Reconciliation Ingestion; Feature: Medication Administration Tracking; Function: Project Longitudinal Medication History; Service: Longitudinal Medication History Query; Function: Correlate Adverse Reaction & Allergy Profile; Service: Allergy & Adverse Reaction Query
BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Enterprise CT2: Interoperability & Exchange (p. 854)	Proposed Care contexts compose qualified information and boundary exchange; this recurrence does not require another record store or specialised capture engine in every setting. Examples: Feature: Medication History Reconciliation Ingestion; Feature: Medication Administration Tracking; Function: Project Longitudinal Medication History; Service: Longitudinal Medication History Query; Function: Correlate Adverse Reaction & Allergy Profile; Service: Allergy & Adverse Reaction Query
BEC CT2: Clinic & Practice Operations (p. 514)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Clinic Session Tracking; Feature: Operational Queue Progression; Function: Manage Clinic & Practice Capacity; Function: Track Clinic Operating Session; Function: Manage Clinic Queue Progression; Service: Clinic Queue Telemetry Service
BEC CT2: Clinic & Practice Operations (p. 514)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Clinic Session Tracking; Feature: Operational Queue Progression; Function: Manage Clinic & Practice Capacity; Function: Track Clinic Operating Session; Function: Manage Clinic Queue Progression; Service: Clinic Queue Telemetry Service
BEC CT2: Clinic & Practice Operations (p. 514)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Clinic Session Tracking; Feature: Operational Queue Progression; Function: Manage Clinic & Practice Capacity; Function: Track Clinic Operating Session; Function: Manage Clinic Queue Progression; Service: Clinic Queue Telemetry Service

From	To	Assertion / rationale
BEC CT2: Ward Operations (p. 524)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Ward Patient Flow Coordination; Feature: Clinical Handover Context Assembly; Function: Manage Ward Capacity; Service: Ward Operational Summary Query; Function: Assemble Ward Handover Context; Feature: Ward Care Capacity Management
BEC CT2: Ward Operations (p. 524)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Ward Patient Flow Coordination; Feature: Clinical Handover Context Assembly; Function: Manage Ward Capacity; Service: Ward Operational Summary Query; Function: Assemble Ward Handover Context; Feature: Ward Care Capacity Management
BEC CT2: Ward Operations (p. 524)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Ward Patient Flow Coordination; Feature: Clinical Handover Context Assembly; Function: Manage Ward Capacity; Service: Ward Operational Summary Query; Function: Assemble Ward Handover Context; Feature: Ward Care Capacity Management
BEC CT2: Theatre Operations (p. 538)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Operating Theatre Case Progression; Feature: Perioperative Resource Notification; Function: Manage Theatre Capacity; Function: Manage Theatre Case Progression; Service: Theatre Case Progression Telemetry; Feature: Theatre Session Capacity Management
BEC CT2: Theatre Operations (p. 538)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Operating Theatre Case Progression; Feature: Perioperative Resource Notification; Function: Manage Theatre Capacity; Function: Manage Theatre Case Progression; Service: Theatre Case Progression Telemetry; Feature: Theatre Session Capacity Management

From	To	Assertion / rationale
BEC CT2: Theatre Operations (p. 538)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Operating Theatre Case Progression; Feature: Perioperative Resource Notification; Function: Manage Theatre Capacity; Function: Manage Theatre Case Progression; Service: Theatre Case Progression Telemetry; Feature: Theatre Session Capacity Management
BEC CT2: Emergency Department Operations (p. 548)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: ED Bed & Bay Tracking; Feature: ED Length of Stay Tracking; Function: Coordinate ED Departmental Capacity; Service: ED Operational Status Query; Feature: Emergency Treatment Capacity Management
BEC CT2: Emergency Department Operations (p. 548)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: ED Bed & Bay Tracking; Feature: ED Length of Stay Tracking; Function: Coordinate ED Departmental Capacity; Service: ED Operational Status Query; Feature: Emergency Treatment Capacity Management
BEC CT2: Emergency Department Operations (p. 548)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: ED Bed & Bay Tracking; Feature: ED Length of Stay Tracking; Function: Coordinate ED Departmental Capacity; Service: ED Operational Status Query; Feature: Emergency Treatment Capacity Management
BEC CT2: Outpatient Operations (p. 561)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Outpatient Arrival Registration; Feature: Post-Clinic Order Coordination; Function: Manage Outpatient Capacity; Service: Ambulatory Session Status Query; Function: Capture Post-Clinic Follow-up Requirements; Feature: Outpatient Appointment Capacity Management

From	To	Assertion / rationale
BEC CT2: Outpatient Operations (p. 561)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Outpatient Arrival Registration; Feature: Post-Clinic Order Coordination; Function: Manage Outpatient Capacity; Service: Ambulatory Session Status Query; Function: Capture Post-Clinic Follow-up Requirements; Feature: Outpatient Appointment Capacity Management
BEC CT2: Outpatient Operations (p. 561)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Outpatient Arrival Registration; Feature: Post-Clinic Order Coordination; Function: Manage Outpatient Capacity; Service: Ambulatory Session Status Query; Function: Capture Post-Clinic Follow-up Requirements; Feature: Outpatient Appointment Capacity Management
BEC CT2: Mobile Staff Management (p. 591)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Mobile Worker Task Dispatch; Feature: Mobile Visit Status Tracking; Function: Manage Mobile Service Capacity; Function: Track Operational Staff Presence; Service: Staff Presence Telemetry Query; Function: Receive Mobile Visit Progression
BEC CT2: Mobile Staff Management (p. 591)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Mobile Worker Task Dispatch; Feature: Mobile Visit Status Tracking; Function: Manage Mobile Service Capacity; Function: Track Operational Staff Presence; Service: Staff Presence Telemetry Query; Function: Receive Mobile Visit Progression
BEC CT2: Mobile Staff Management (p. 591)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Mobile Worker Task Dispatch; Feature: Mobile Visit Status Tracking; Function: Manage Mobile Service Capacity; Function: Track Operational Staff Presence; Service: Staff Presence Telemetry Query; Function: Receive Mobile Visit Progression

From	To	Assertion / rationale
BEC CT2: Service Capacity Management (p. 581)	Enterprise CT2: Context Management (p. 839)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Operational Capacity Metric Aggregation; Function: Aggregate Service Capacity Telemetry; Service: Regional Health System Capacity Query
BEC CT2: Service Capacity Management (p. 581)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Operational Capacity Metric Aggregation; Function: Aggregate Service Capacity Telemetry; Service: Regional Health System Capacity Query
BEC CT2: Service Capacity Management (p. 581)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Local flow/capacity changes need context-qualified state and notifications; common mechanisms do not transfer capacity semantics to a central owner. Examples: Feature: Operational Capacity Metric Aggregation; Function: Aggregate Service Capacity Telemetry; Service: Regional Health System Capacity Query
BEC CT2: Work Allocation & Dispatch (p. 607)	Enterprise CT2: Context Management (p. 839)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Work Item Instantiation; Feature: Worker Matching & Allocation; Feature: Work Dispatch Delivery; Feature: Work Progress Oversight & Escalation; Function: Dispatch Operational Work Order; Service: Work Dispatch Service
BEC CT2: Work Allocation & Dispatch (p. 607)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Work Item Instantiation; Feature: Worker Matching & Allocation; Feature: Work Dispatch Delivery; Feature: Work Progress Oversight & Escalation; Function: Dispatch Operational Work Order; Service: Work Dispatch Service
BEC CT2: Work Allocation & Dispatch (p. 607)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Work Item Instantiation; Feature: Worker Matching & Allocation; Feature: Work Dispatch Delivery; Feature: Work Progress Oversight & Escalation; Function: Dispatch Operational Work Order; Service: Work Dispatch Service

From	To	Assertion / rationale
BEC CT2: Patient Transport (p. 632)	Enterprise CT2: Context Management (p. 839)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Transport Request Ingestion; Feature: Transport Dispatch & Progress Tracking; Function: Manage Patient Transport Progression; Service: Patient Transport Tracking Service
BEC CT2: Patient Transport (p. 632)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Transport Request Ingestion; Feature: Transport Dispatch & Progress Tracking; Function: Manage Patient Transport Progression; Service: Patient Transport Tracking Service
BEC CT2: Patient Transport (p. 632)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Transport Request Ingestion; Feature: Transport Dispatch & Progress Tracking; Function: Manage Patient Transport Progression; Service: Patient Transport Tracking Service
BEC CT2: Clinical Logistics Coordination (p. 639)	Enterprise CT2: Context Management (p. 839)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Pathology Specimen Transport Tracking; Function: Manage Specimen Transport Progression; Service: Specimen Transit Tracking Service
BEC CT2: Clinical Logistics Coordination (p. 639)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Pathology Specimen Transport Tracking; Function: Manage Specimen Transport Progression; Service: Specimen Transit Tracking Service
BEC CT2: Clinical Logistics Coordination (p. 639)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Pathology Specimen Transport Tracking; Function: Manage Specimen Transport Progression; Service: Specimen Transit Tracking Service

From	To	Assertion / rationale
BEC CT2: Discharge Management (p. 645)	Enterprise CT2: Context Management (p. 839)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Discharge Milestone Tracking; Feature: Discharge Readiness & Coordination Oversight; Feature: Discharge Notification Dispatch; Function: Manage Discharge Coordination Progression; Service: Discharge Readiness Telemetry
BEC CT2: Discharge Management (p. 645)	Enterprise CT2: Activity & Execution (p. 857)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Discharge Milestone Tracking; Feature: Discharge Readiness & Coordination Oversight; Feature: Discharge Notification Dispatch; Function: Manage Discharge Coordination Progression; Service: Discharge Readiness Telemetry
BEC CT2: Discharge Management (p. 645)	Enterprise CT2: Operational Assurance (p. 873)	Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Discharge Milestone Tracking; Feature: Discharge Readiness & Coordination Oversight; Feature: Discharge Notification Dispatch; Function: Manage Discharge Coordination Progression; Service: Discharge Readiness Telemetry
BEC CT3: Workflow & Activity Coordination (p. 891)	Enterprise CT2: Context Management (p. 839)	Historical assertion / Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Work Order Progression; Feature: To Do Decision & Approval Coordination; Feature: Synthetic Task Orchestration; Feature: Workflow Timeout & Escalation Oversight; Function: Coordinate Work Order; Service: Work Order Coordination Service
BEC CT3: Workflow & Activity Coordination (p. 891)	Enterprise CT2: Activity & Execution (p. 857)	Historical assertion / Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Work Order Progression; Feature: To Do Decision & Approval Coordination; Feature: Synthetic Task Orchestration; Feature: Workflow Timeout & Escalation Oversight; Function: Coordinate Work Order; Service: Work Order Coordination Service

From	To	Assertion / rationale
BEC CT3: Workflow & Activity Coordination (p. 891)	Enterprise CT2: Operational Assurance (p. 873)	Historical assertion / Proposed Operational activity needs eligible progression, effect standing and recoverable execution under context; clinical work and source clearances retain their authority. Examples: Feature: Work Order Progression; Feature: To Do Decision & Approval Coordination; Feature: Synthetic Task Orchestration; Feature: Workflow Timeout & Escalation Oversight; Function: Coordinate Work Order; Service: Work Order Coordination Service
BEC CT2: Clinical Collaboration (p. 756)	Enterprise CT2: Interaction & Experience (p. 862)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Care-Team Clinical Collaboration Coordination; Feature: In-Conversation LHR Query Resolution; Feature: Zero-PHI Collaboration Metadata Governance; Function: Coordinate Care-Team Collaboration; Service: Clinical Collaboration Space Service; Function: Resolve LHR Query within Collaboration
BEC CT2: Clinical Collaboration (p. 756)	Enterprise CT2: Policy & Control (p. 865)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Care-Team Clinical Collaboration Coordination; Feature: In-Conversation LHR Query Resolution; Feature: Zero-PHI Collaboration Metadata Governance; Function: Coordinate Care-Team Collaboration; Service: Clinical Collaboration Space Service; Function: Resolve LHR Query within Collaboration
BEC CT2: Clinical Collaboration (p. 756)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Care-Team Clinical Collaboration Coordination; Feature: In-Conversation LHR Query Resolution; Feature: Zero-PHI Collaboration Metadata Governance; Function: Coordinate Care-Team Collaboration; Service: Clinical Collaboration Space Service; Function: Resolve LHR Query within Collaboration
BEC CT2: General Collaboration (p. 768)	Enterprise CT2: Interaction & Experience (p. 862)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Operational Team Chat Enablement; Function: Coordinate Operational Collaboration; Service: Operational Collaboration Service

From	To	Assertion / rationale
BEC CT2: General Collaboration (p. 768)	Enterprise CT2: Policy & Control (p. 865)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Operational Team Chat Enablement; Function: Coordinate Operational Collaboration; Service: Operational Collaboration Service
BEC CT2: General Collaboration (p. 768)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Operational Team Chat Enablement; Function: Coordinate Operational Collaboration; Service: Operational Collaboration Service
BEC CT2: Presentation Services (p. 786)	Enterprise CT2: Interaction & Experience (p. 862)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Longitudinal Clinical & Operational Presentation; Function: Present Contextual Health and Operational Information; Service: Contextual Presentation Service
BEC CT2: Presentation Services (p. 786)	Enterprise CT2: Policy & Control (p. 865)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Longitudinal Clinical & Operational Presentation; Function: Present Contextual Health and Operational Information; Service: Contextual Presentation Service
BEC CT2: Presentation Services (p. 786)	Enterprise CT2: Event & Subscription (p. 856)	Proposed Human interaction and notifications consume governed context without becoming clinical truth, policy ownership or a central collaboration service. Examples: Feature: Longitudinal Clinical & Operational Presentation; Function: Present Contextual Health and Operational Information; Service: Contextual Presentation Service
BEC CT2: Information Design Governance (p. 792)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Information Standards & Semantic Governance; Feature: Semantic Conformance Verification; Function: Govern Information Semantics; Service: Semantic Model Definition Service; Function: Govern Information Definition; Service: Information Dictionary Query

From	To	Assertion / rationale
BEC CT2: Information Design Governance (p. 792)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Information Standards & Semantic Governance; Feature: Semantic Conformance Verification; Function: Govern Information Semantics; Service: Semantic Model Definition Service; Function: Govern Information Definition; Service: Information Dictionary Query
BEC CT2: Information Design Governance (p. 792)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Information Standards & Semantic Governance; Feature: Semantic Conformance Verification; Function: Govern Information Semantics; Service: Semantic Model Definition Service; Function: Govern Information Definition; Service: Information Dictionary Query
BEC CT2: Clinical Knowledge Services (p. 742)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service
BEC CT2: Clinical Knowledge Services (p. 742)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service
BEC CT2: Clinical Knowledge Services (p. 742)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Clinical Concept Resolution; Feature: Bidirectional Terminology Mapping; Function: Resolve Clinical Concept; Service: Terminology Concept Resolution Service; Function: Map Clinical Concept; Service: Terminology Mapping Service

From	To	Assertion / rationale
BEC CT3: Governed Reference Dataset Administration (p. 748)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query
BEC CT3: Governed Reference Dataset Administration (p. 748)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query
BEC CT3: Governed Reference Dataset Administration (p. 748)	Enterprise CT2: Provenance & Traceability (p. 869)	Proposed Definition, binding and mutable reference-content governance share conformance and attribution while retaining separate lifecycle meanings. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query
BEC CT3: Information Authority Representation (p. 959)	Enterprise CT2: Information Authority Qualification (p. 877)	Historical assertion / Proposed Source standing recurs in competing identity assertions, clinical synthesis, reference content and evidence use; persistence success and freshness are insufficient substitutes. Examples: Feature: Information Authority & Source Standing Representation; Function: Qualify Information Authority Standing
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed Source standing recurs in competing identity assertions, clinical synthesis, reference content and evidence use; persistence success and freshness are insufficient substitutes. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BEC CT3: Person Identity Administration (p. 214)	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed Source standing recurs in competing identity assertions, clinical synthesis, reference content and evidence use; persistence success and freshness are insufficient substitutes. Examples: Feature: Identifier Resolution; Feature: Cross-Authority Identifier Correlation; Feature: Identity Alias Association; Feature: Governed Identity Correction; Function: Resolve Person Identifier; Service: Person Identifier Resolution

From	To	Assertion / rationale
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Information Authority Qualification (p. 877)	Historical assertion / Proposed Source standing recurs in competing identity assertions, clinical synthesis, reference content and evidence use; persistence success and freshness are insufficient substitutes. Examples: Function: Establish Assurance Context; Function: Assess Assurance Evidence; Function: Adjudicate Assurance Assessment; Service: Request Service Assurance; Service: Request Service Assurance Status; Service: Communicate Service Assurance Outcome
BEC CT3: Governed Reference Dataset Administration (p. 748)	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed Source standing recurs in competing identity assertions, clinical synthesis, reference content and evidence use; persistence success and freshness are insufficient substitutes. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query
BEC CT3: Linked Information Curation (p. 663)	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed Documents and collaboration references can require payload resolution while logical ownership, collaboration-native media and physical custody stay distinct. Examples: Feature: Binary Content Resolution & Assembly; Function: Resolve Associated Binary Content; Function: Assemble Associated Binary Content; Service: Associated Binary Content Retrieval
BEC CT2: Clinical Record Administration (p. 443)	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed Documents and collaboration references can require payload resolution while logical ownership, collaboration-native media and physical custody stay distinct. Examples: Feature: Clinical Document Ingestion; Feature: Document Versioning & Supersession; Feature: Document Metadata Indexing; Function: Receive Clinical Document; Service: Clinical Document Ingress; Function: Govern Clinical Document Lifecycle
BEC CT2: Clinical Collaboration (p. 756)	Enterprise CT3: Binary Content Association & Resolution (p. 848)	Proposed Documents and collaboration references can require payload resolution while logical ownership, collaboration-native media and physical custody stay distinct. Examples: Feature: Care-Team Clinical Collaboration Coordination; Feature: In-Conversation LHR Query Resolution; Feature: Zero-PHI Collaboration Metadata Governance; Function: Coordinate Care-Team Collaboration; Service: Clinical Collaboration Space Service; Function: Resolve LHR Query within Collaboration

From	To	Assertion / rationale
BEC CT3: Significant Activity Audit Capture (p. 961)	Enterprise CT3: Governed Activity Audit Evidence (p. 872)	Historical assertion / Proposed Required evidence of governed actions recurs across policy, disclosure and communication; only mandatory authoritative capture has the specifically established intrinsic authority. Examples: Feature: Mandatory Significant Activity Audit Capture; Function: Capture Required Significant Audit Evidence; Service: Mandatory Audit Evidence Capture
BEC CT2: Health Information Control (p. 715)	Enterprise CT3: Governed Activity Audit Evidence (p. 872)	Proposed Required evidence of governed actions recurs across policy, disclosure and communication; only mandatory authoritative capture has the specifically established intrinsic authority. Examples: Feature: Health Information Access & Consent Control; Feature: Security Context Propagation; Feature: Non-PHI Compliance Auditing; Function: Evaluate Information Access Authority; Service: Policy Evaluation & Authorisation Service; Function: Evaluate Consent Constraint
BEC CT2: Clinical Communication Administration (p. 476)	Enterprise CT3: Governed Activity Audit Evidence (p. 872)	Proposed Required evidence of governed actions recurs across policy, disclosure and communication; only mandatory authoritative capture has the specifically established intrinsic authority. Examples: Feature: Clinical Communication Distribution; Feature: Delivery Acknowledgement Tracking; Feature: Communication Audit Logging; Function: Distribute Clinical Communication; Service: Secure Clinical Message Dispatch; Function: Track Business Delivery Acknowledgement
BEC CT3: Data Provenance Capture (p. 733)	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed Correction history, source attribution and derived clinical synthesis require meaningful lineage rather than infrastructure traces. Examples: Feature: Information Transformation Lineage; Function: Record Significant Information Lineage
BEC CT3: Person Identity Administration (p. 214)	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed Correction history, source attribution and derived clinical synthesis require meaningful lineage rather than infrastructure traces. Examples: Feature: Identifier Resolution; Feature: Cross-Authority Identifier Correlation; Feature: Identity Alias Association; Feature: Governed Identity Correction; Function: Resolve Person Identifier; Service: Person Identifier Resolution
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed Correction history, source attribution and derived clinical synthesis require meaningful lineage rather than infrastructure traces. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record

From	To	Assertion / rationale
BEC CT3: Entity-Specific Operational Coordination (p. 963)	Enterprise CT3: Entity-Relative Operational Progression (p. 861)	Historical assertion / Proposed Patient progression, bed turnover and entity-specific operational activity benefit from a coherent entity-relative execution frame; this is a proposed reusable contribution rather than component allocation. Examples: Feature: Entity-Relative Activity Coordination; Feature: Entity Behaviour Failure & Recovery Governance; Function: Coordinate Entity-Relative Input; Service: Entity Operational Input Submission
BEC CT2: Discharge Management (p. 645)	Enterprise CT3: Entity-Relative Operational Progression (p. 861)	Proposed Patient progression, bed turnover and entity-specific operational activity benefit from a coherent entity-relative execution frame; this is a proposed reusable contribution rather than component allocation. Examples: Feature: Discharge Milestone Tracking; Feature: Discharge Readiness & Coordination Oversight; Feature: Discharge Notification Dispatch; Function: Manage Discharge Coordination Progression; Service: Discharge Readiness Telemetry
BEC CT2: Bed & Care-Place Management (p. 569)	Enterprise CT3: Entity-Relative Operational Progression (p. 861)	Proposed Patient progression, bed turnover and entity-specific operational activity benefit from a coherent entity-relative execution frame; this is a proposed reusable contribution rather than component allocation. Examples: Feature: Bed Availability Tracking; Feature: Bed State Progression; Feature: Bed Readiness Notification; Function: Maintain Operational Bed State; Service: Bed Availability Query; Function: Manage Bed Turnover Progression
BEC CT3: Governed Operational Activity Design (p. 964)	Enterprise CT3: Operational Fulfilment Definition (p. 859)	Historical assertion / Proposed Defined fulfilment context and execution control recur in human work, human decisions and synthetic processing, while remaining different abilities. Examples: Feature: Operational Activity Context Definition; Feature: Activity Outcome & Continuation Rules; Function: Maintain Operational Fulfilment Definition
BEC CT3: Governed Operational Activity Design (p. 964)	Enterprise CT3: Governed Activity Execution Control (p. 860)	Historical assertion / Proposed Defined fulfilment context and execution control recur in human work, human decisions and synthetic processing, while remaining different abilities. Examples: Feature: Operational Activity Context Definition; Feature: Activity Outcome & Continuation Rules; Function: Maintain Operational Fulfilment Definition

From	To	Assertion / rationale
BEC CT3: Workflow & Activity Coordination (p. 891)	Enterprise CT3: Operational Fulfilment Definition (p. 859)	Historical assertion / Proposed Defined fulfilment context and execution control recur in human work, human decisions and synthetic processing, while remaining different abilities. Examples: Feature: Work Order Progression; Feature: To Do Decision & Approval Coordination; Feature: Synthetic Task Orchestration; Feature: Workflow Timeout & Escalation Oversight; Function: Coordinate Work Order; Service: Work Order Coordination Service
BEC CT3: Workflow & Activity Coordination (p. 891)	Enterprise CT3: Governed Activity Execution Control (p. 860)	Historical assertion / Proposed Defined fulfilment context and execution control recur in human work, human decisions and synthetic processing, while remaining different abilities. Examples: Feature: Work Order Progression; Feature: To Do Decision & Approval Coordination; Feature: Synthetic Task Orchestration; Feature: Workflow Timeout & Escalation Oversight; Function: Coordinate Work Order; Service: Work Order Coordination Service
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT3: Active Information Availability (p. 846)	Proposed Both clinical and reference information require responsive use and authoritative revision establishment, with separate trust/concurrency domains. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BEC CT2: Patient Clinical Record (p. 656)	Enterprise CT3: Authoritative Durable State Establishment (p. 847)	Proposed Both clinical and reference information require responsive use and authoritative revision establishment, with separate trust/concurrency domains. Examples: Feature: Longitudinal Health Record (LHR) Assembly; Feature: Active Clinical Record Access; Feature: Durable Clinical Record Preservation; Function: Assemble Longitudinal Clinical Record; Service: Longitudinal Clinical Record Query; Function: Maintain Active Clinical Record
BEC CT3: Governed Reference Dataset Administration (p. 748)	Enterprise CT3: Active Information Availability (p. 846)	Proposed Both clinical and reference information require responsive use and authoritative revision establishment, with separate trust/concurrency domains. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query

From	To	Assertion / rationale
BEC CT3: Governed Reference Dataset Administration (p. 748)	Enterprise CT3: Authoritative Durable State Establishment (p. 847)	Proposed Both clinical and reference information require responsive use and authoritative revision establishment, with separate trust/concurrency domains. Examples: Feature: Governed Dataset Content Revision; Function: Accept Governed Dataset Revision; Service: Governed Reference Dataset Content Query

32.13. Enterprise Contribution

From	To	Assertion / rationale
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Established Supporting assurance-strategy:60 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Managed Entity & Relationship (p. 838)	Historical assertion / Established Supporting assurance-strategy:60 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Context Management (p. 839)	Established Supporting assurance-strategy:61 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Context Management (p. 839)	Established Supporting assurance-strategy:61 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Context Management (p. 839)	Historical assertion / Established Direct assurance-strategy:61 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Established Supporting assurance-strategy:62 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Established Direct assurance-strategy:62 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Managed State & Lifecycle (p. 842)	Historical assertion / Established Direct assurance-strategy:62 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Information Management (p. 844)	Established Direct assurance-strategy:63 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Information Management (p. 844)	Established Direct assurance-strategy:63 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Information Management (p. 844)	Historical assertion / Established Direct assurance-strategy:63 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Search & Discovery (p. 849)	Established Supporting assurance-strategy:64 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Search & Discovery (p. 849)	Established Supporting assurance-strategy:64 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Search & Discovery (p. 849)	Historical assertion / Established Supporting assurance-strategy:64 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Policy & Control (p. 865)	Established Direct assurance-strategy:65 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Policy & Control (p. 865)	Established Direct assurance-strategy:65 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Policy & Control (p. 865)	Historical assertion / Established Supporting assurance-strategy:65 (p. 1408)

From	To	Assertion / rationale
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Provenance & Traceability (p. 869)	Established Supporting assurance-strategy:66 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Provenance & Traceability (p. 869)	Established Supporting assurance-strategy:66 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Provenance & Traceability (p. 869)	Historical assertion / Established Direct assurance-strategy:66 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Interoperability & Exchange (p. 854)	Established Supporting assurance-strategy:67 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Interoperability & Exchange (p. 854)	Historical assertion / Established Supporting assurance-strategy:67 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Event & Subscription (p. 856)	Established Supporting assurance-strategy:68 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Event & Subscription (p. 856)	Historical assertion / Established Direct assurance-strategy:68 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Activity & Execution (p. 857)	Established Supporting assurance-strategy:69 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Activity & Execution (p. 857)	Historical assertion / Established Direct assurance-strategy:69 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Interaction & Experience (p. 862)	Established Supporting assurance-strategy:70 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Interaction & Experience (p. 862)	Established Supporting assurance-strategy:70 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Interaction & Experience (p. 862)	Historical assertion / Established Supporting assurance-strategy:70 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Operational Assurance (p. 873)	Established Supporting assurance-strategy:71 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Operational Assurance (p. 873)	Established Supporting assurance-strategy:71 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Operational Assurance (p. 873)	Historical assertion / Established Direct, limited to generic operational/processing assurance contribution assurance-strategy:71 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Established Supporting assurance-strategy:72 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Established Direct assurance-strategy:72 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Historical assertion / Established Supporting assurance-strategy:72 (p. 1408)
BEC CT3: Assurance Design (p. 824)	Enterprise CT2: Service Guardian (p. 875)	Established Direct assurance-strategy:73 (p. 1408)
BEC CT3: Assurance Criteria Administration (p. 828)	Enterprise CT2: Service Guardian (p. 875)	Established Direct assurance-strategy:73 (p. 1408)
BEC CT3: Governed Assurance (p. 898)	Enterprise CT2: Service Guardian (p. 875)	Historical assertion / Established Direct assurance-strategy:73 (p. 1408)

32.14. Purpose Context

From	To	Assertion / rationale
Feature: Authorised Research Release Assembly (p. 739)	BEC CT2: Research Information Curation (p. 812)	Proposed The existing purpose-specific Feature uses shared Information Product Administration framing while retaining its research, population-health or planning purpose. This does not decompose the purpose CT2 or confer analytical authority.
Feature: Population Notification Obligation Tracking (p. 740)	BEC CT2: Population Health Information Reporting (p. 817)	Proposed The existing purpose-specific Feature uses shared Information Product Administration framing while retaining its research, population-health or planning purpose. This does not decompose the purpose CT2 or confer analytical authority.
Feature: Demand & Capacity Planning Projection (p. 741)	BEC CT2: Service Planning Data Management (p. 818)	Proposed The existing purpose-specific Feature uses shared Information Product Administration framing while retaining its research, population-health or planning purpose. This does not decompose the purpose CT2 or confer analytical authority.

32.15. Established absence constraints

The approved assurance matrix explicitly records No Material Role for these pairs; this is preserved rather than filled by analogy.

BEC	EC	Constraint
BEC CT3: Assurance Criteria Administration	Enterprise CT2: Managed Entity & Relationship	No Material Role
BEC CT3: Assurance Design	Enterprise CT2: Interoperability & Exchange	No Material Role
BEC CT3: Assurance Criteria Administration	Enterprise CT2: Event & Subscription	No Material Role
BEC CT3: Assurance Criteria Administration	Enterprise CT2: Activity & Execution	No Material Role

33. Contextual views and prior descriptions

These views provide navigation across the proposed hierarchy. Membership remains different from decomposition. In entity information, distinct named descriptions become Proposed CT3s. Elsewhere the Canonical responsibility itself usually provides CT3 framing, so an old description is not mechanically made another tier.

33.1. Entity Management

Source contextual description	Current responsibility frame	Standing
Person Identity	BEC CT3: Person Identity Administration (p. 214)	CT3 / Canonical; placement Proposed
Healthcare Subject	BEC CT3: Healthcare Subject Profile Administration (p. 226)	CT3 / Canonical; placement Proposed
Client Relationship	BEC CT3: Client Relationship Administration (p. 232)	CT3 / Canonical; placement Proposed
Client Privacy	BEC CT3: Client Privacy Guardrail Management (p. 237)	CT3 / Canonical; placement Proposed
Practitioner	BEC CT3: Practitioner Profile Administration (p. 245)	CT3 / Proposed; placement Proposed
Practitioner Role	BEC CT3: Practitioner Role Administration (p. 249)	CT3 / Proposed; placement Proposed
Provider Relationship	BEC CT3: Practitioner Role Administration (p. 249)	CT3 / Proposed; placement Proposed
Provider Engagement	BEC CT3: Provider Clinical Qualification Tracking (p. 623)	CT3 / Proposed; placement Proposed
Healthcare Organisation	BEC CT3: Healthcare Organisation Profile Administration (p. 265)	CT3 / Proposed; placement Proposed
Organisational Structure	BEC CT3: Healthcare Organisation Profile Administration (p. 265)	CT3 / Proposed; placement Proposed
Organisation Contact	BEC CT3: Healthcare Organisation Profile Administration (p. 265)	CT3 / Proposed; placement Proposed
Healthcare Location	BEC CT3: Healthcare Location Profile Administration (p. 277)	CT3 / Proposed; placement Proposed
Location Structure	BEC CT3: Healthcare Location Profile Administration (p. 277)	CT3 / Proposed; placement Proposed
Care-Place Definition	BEC CT3: Care-Place Definition Administration (p. 281)	CT3 / Proposed; placement Proposed
Healthcare Service Definition	BEC CT3: Healthcare Service Profile Administration (p. 289)	CT3 / Proposed; placement Proposed
Service Provision Relationship	BEC CT3: Healthcare Service Profile Administration (p. 289)	CT3 / Proposed; placement Proposed
Service Availability Definition	BEC CT3: Healthcare Service Profile Administration (p. 289)	CT3 / Proposed; placement Proposed
Service Eligibility Definition	BEC CT3: Healthcare Service Profile Administration (p. 289)	CT3 / Proposed; placement Proposed
Health Product Definition	BEC CT3: Health Materiel Profile Administration (p. 301)	CT3 / Proposed; placement Proposed
Product Formulary & Catalogue	BEC CT3: Health Materiel Catalogue & Formulary Administration (p. 302)	CT3 / Proposed; placement Proposed
Product Batch & Serial Tracking	BEC CT3: Health Materiel Traceability Administration (p. 304)	CT3 / Proposed; placement Proposed

Source contextual description	Current responsibility frame	Standing
Device Definition	BEC CT3: Clinical Device Profile Administration (p. 307)	CT3 / Proposed; placement Proposed
Device Instance	BEC CT3: Clinical Device Profile Administration (p. 307)	CT3 / Proposed; placement Proposed
Device Association	BEC CT3: Device Utilisation Administration (p. 311)	CT3 / Proposed; placement Proposed
Device Connectivity	BEC CT3: Device OA&M Metadata Management (p. 316)	CT3 / Proposed; placement Proposed

33.2. Service Administration

Source contextual description	Current responsibility frame	Standing
Client Intake & Demographic Update	BEC CT3: Client Intake & Demographic Update (p. 336)	CT3 / Proposed; placement Proposed
Referral Intake & Progression	BEC CT3: Referral Intake & Progression (p. 344)	CT3 / Proposed; placement Proposed
Appointment & Booking Administration	BEC CT3: Appointment & Booking Administration (p. 357)	CT3 / Proposed; placement Proposed
Encounter Lifecycle Management	BEC CT3: Encounter Lifecycle Management (p. 371)	CT3 / Proposed; placement Proposed
Closed-Loop Order Progression	BEC CT3: Closed-Loop Order Progression (p. 385)	CT3 / Proposed; placement Proposed
Diagnostic Workflow Management	BEC CT3: Diagnostic Workflow Management (p. 408)	CT3 / Proposed; placement Proposed
Medication Order & Dispense Management	BEC CT3: Medication Order & Dispense Management (p. 426)	CT3 / Proposed; placement Proposed
Procedure & Surgical Administration	BEC CT3: Procedure & Surgical Administration (p. 437)	CT3 / Proposed; placement Proposed
Clinical Documentation Lifecycle	BEC CT3: Clinical Documentation Lifecycle Administration (p. 445)	CT3 / Proposed; placement Proposed
Care Plan Coordination	BEC CT3: Care Plan Lifecycle & Coordination (p. 457)	CT3 / Proposed; placement Proposed
Secure Clinical Communication	BEC CT3: Secure Clinical Communication (p. 478)	CT3 / Proposed; placement Proposed

33.3. Service Delivery

Source contextual description	Current responsibility frame	Standing
Primary Care Coordination	BEC CT3: Primary Care Coordination (p. 669)	CT3 / Canonical; placement Proposed
Acute Care Monitoring & Event Coordination	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	CT3 / Canonical; placement Proposed
Emergency Care Progression	BEC CT3: Emergency Care Progression (p. 554)	CT3 / Canonical; placement Proposed
Multidisciplinary Inpatient Care Coordination	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	CT3 / Canonical; placement Proposed
Diagnostic Service Integration	BEC CT3: Diagnostic Service Integration (p. 401)	CT3 / Canonical; placement Proposed
Medication Therapy Provision and Monitoring	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	CT3 / Canonical; placement Proposed
Screening and Immunisation Enablement	BEC CT3: Screening and Immunisation Enablement (p. 329)	CT3 / Canonical; placement Proposed
Community Care Coordination	BEC CT3: Community Care Coordination (p. 460)	CT3 / Canonical; placement Proposed

Source contextual description	Current responsibility frame	Standing
Outreach Care Continuity	BEC CT3: Outreach Care Continuity (p. 466)	CT3 / Canonical; placement Proposed
Remote Patient Monitoring	BEC CT3: Remote Patient Monitoring (p. 675)	CT3 / Canonical; placement Proposed

33.4. Health Service Operations

Source contextual description	Current responsibility frame	Standing
Session & Queue Coordination	BEC CT3: Session & Queue Coordination (p. 515)	CT3 / Proposed; placement Proposed
Ward Logistics Coordination	BEC CT3: Ward Logistics Coordination (p. 525)	CT3 / Proposed; placement Proposed
Perioperative Logistics	BEC CT3: Perioperative Logistics Coordination (p. 540)	CT3 / Proposed; placement Proposed
ED Operational Coordination	BEC CT3: Emergency Department Operational Coordination (p. 549)	CT3 / Proposed; placement Proposed
Outpatient Flow Enablement	BEC CT3: Outpatient Flow Coordination (p. 563)	CT3 / Proposed; placement Proposed
Bed State & Turnover Management	BEC CT3: Bed & Care-Place State Administration (p. 571)	CT3 / Proposed; placement Proposed
Specialized Equipment Coordination	BEC CT3: Device Utilisation Administration (p. 311)	CT3 / Proposed; placement Proposed
Facility Demand & Capacity Telemetry	BEC CT3: Service Demand & Capacity Coordination (p. 582)	CT3 / Proposed; placement Proposed
Mobile Worker Dispatch & Safety	BEC CT3: Mobile Worker Dispatch & Safety Coordination (p. 593)	CT3 / Proposed; placement Proposed
On-Call Directory Resolution	BEC CT3: On-Call Availability & Contact Resolution (p. 603)	CT3 / Proposed; placement Proposed
Operational Work Assignment & Supervision	BEC CT3: Operational Work Assignment & Dispatch Coordination (p. 608)	CT3 / Proposed; placement Proposed
Credential Verification Checkpoint	BEC CT3: Provider Clinical Qualification Tracking (p. 623)	CT3 / Proposed; placement Proposed
Patient Transfer Coordination	BEC CT3: Patient Transfer Coordination (p. 634)	CT3 / Proposed; placement Proposed
Specimen & Supply Courier Tracking	BEC CT3: Specimen Movement Traceability (p. 642)	CT3 / Proposed; placement Proposed
Discharge Progression & Coordination	BEC CT3: Discharge Progression & Coordination (p. 647)	CT3 / Proposed; placement Proposed

33.5. Intrinsic / Shared Enablement

Source contextual description	Current responsibility frame	Standing
Longitudinal Clinical Record Governance	BEC CT3: Longitudinal Clinical Record Administration (p. 658)	CT3 / Proposed; placement Proposed
Multi-Model Information Exchange	BEC CT3: HIE Implementation Management (p. 689)	CT3 / Proposed; placement Proposed
Governed Query & Search	BEC CT3: Governed Health Information Retrieval (p. 703)	CT3 / Proposed; placement Proposed
Boundary Protocol Mediation	BEC CT3: Health Information Communication (p. 890)	CT3 / Canonical; placement Proposed
Policy & Security Governance	BEC CT2: Health Information Control (p. 715)	CT2 / Canonical; placement Proposed
Terminology & Reference Mapping	BEC CT3: Governed Reference Dataset Administration (p. 748)	CT3 / Proposed; placement Proposed

Source contextual description	Current responsibility frame	Standing
LHR-Aware Multidisciplinary Collaboration	BEC CT3: Clinical Context Collaboration (p. 758)	CT3 / Proposed; placement Proposed
Operational Messaging	BEC CT3: Operational Collaboration (p. 770)	CT3 / Proposed; placement Proposed
Multi-Work-Unit Orchestration	BEC CT3: Workflow & Activity Coordination (p. 891)	CT3 / Canonical; placement Proposed
Operational & Clinical Calendar Sync	BEC CT3: Operational & Clinical Calendar Coordination (p. 780)	CT3 / Proposed; placement Proposed
Decoupled Human Interaction & Dashboards	BEC CT3: Contextual Information Presentation (p. 788)	CT3 / Proposed; placement Proposed
Canonical Semantics & Schema Governance	BEC CT3: Enterprise Canonical/Reusable Model Design (p. 794)	CT3 / Proposed; placement Proposed

34. Identity and change registers

Reference IDs and LaTeX labels are stable. Structural IDs describe current proposed architecture; ID aliases are allocated historical values resolving directly to the same element. Old v0.3 working IDs are provenance locators, not identity aliases.

Reference / structural identity	Element / page	Element / path standing
EC-01 CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-01	Enterprise CT2: Managed Entity & Relationship (p. 838)	Canonical / Proposed
EC-02 CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-02	Enterprise CT2: Context Management (p. 839)	Canonical / Proposed
EC-03 CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-03	Enterprise CT2: Managed State & Lifecycle (p. 842)	Canonical / Proposed
EC-04 CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-04	Enterprise CT2: Information Management (p. 844)	Canonical / Proposed
EC-05 CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-05	Enterprise CT2: Search & Discovery (p. 849)	Canonical / Proposed
EC-06 CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06	Enterprise CT2: Policy & Control (p. 865)	Canonical / Proposed
EC-07 CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-07	Enterprise CT2: Provenance & Traceability (p. 869)	Canonical / Proposed
EC-08 CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-08	Enterprise CT2: Interoperability & Exchange (p. 854)	Canonical / Proposed
EC-09 CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-09	Enterprise CT2: Event & Subscription (p. 856)	Canonical / Proposed
EC-10 CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10	Enterprise CT2: Activity & Execution (p. 857)	Canonical / Proposed
EC-11 CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-11	Enterprise CT2: Interaction & Experience (p. 862)	Canonical / Proposed
EC-12 CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-12	Enterprise CT2: Operational Assurance (p. 873)	Canonical / Proposed
EC-13 CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-13	Enterprise CT2: Semantic Governance & Conformance (p. 851)	Canonical / Proposed
EC-14 CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-14	Enterprise CT2: Service Guardian (p. 875)	Canonical / Proposed
FEAT-EM-01 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-01	Feature: Identifier Resolution (p. 216)	Canonical / Proposed

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FEAT-EM-02 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-02	Feature: Cross-Authority Identifier Correlation (p. 217)	Canonical / Proposed
FEAT-EM-03 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-03	Feature: Identity Alias Association (p. 218)	Canonical / Proposed
FEAT-EM-04 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-EM-04	Feature: Governed Identity Correction (p. 218)	Canonical / Proposed
FEAT-EM-05 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FEAT-EM-05	Feature: Subject Context Binding (p. 227)	Canonical / Proposed
FEAT-EM-06 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FEAT-EM-06	Feature: Demographic Context Governance (p. 228)	Canonical / Proposed
FEAT-EM-07 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FEAT-EM-07	Feature: Next-of-Kin & Guardian Association (p. 233)	Canonical / Proposed
FEAT-EM-08 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FEAT-EM-08	Feature: Relationship Validity Tracking (p. 234)	Canonical / Proposed
FEAT-EM-09 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FEAT-EM-09	Feature: Consent Directive Evaluation (p. 239)	Canonical / Proposed
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FEAT-EM-11 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER.FEAT-EM-11	Feature: National Practitioner Verification (p. 246)	Canonical / Proposed
FEAT-EM-12 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER.FEAT-EM-12	Feature: Practitioner Profile Governance (p. 247)	Canonical / Proposed
FEAT-EM-13 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE.FEAT-EM-13	Feature: Role & Specialty Modeling (p. 251)	Canonical / Proposed
FEAT-EM-14 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE.FEAT-EM-14	Feature: Electronic Endpoint Binding (p. 252)	Canonical / Proposed
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FEAT-EM-17 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-17	Feature: National Organisation Verification (p. 267)	Canonical / Proposed
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FEAT-EM-19 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-19	Feature: Hierarchical Structure Navigation (p. 268)	Canonical / Proposed
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FEAT-EM-21 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION.FEAT-EM-21	Feature: Location Geospatial & Address Resolution (p. 279)	Canonical / Proposed
FEAT-EM-22 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION.FEAT-EM-22	Feature: Physical Hierarchy Modeling (p. 280)	Canonical / Proposed
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FEAT-EM-24 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-24	Feature: Service Catalogue Publishing (p. 291)	Canonical / Proposed
FEAT-EM-25 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-25	Feature: Service-Location-Provider Binding (p. 291)	Canonical / Proposed
FEAT-EM-26 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-26	Feature: Operating Schedule Specification (p. 292)	Canonical / Proposed
FEAT-EM-27 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-27	Feature: Eligibility & Catchment Specification (p. 293)	Canonical / Proposed

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FEAT-EM-28 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION.FEAT-EM-28	Feature: Device Type Specification (p. 309)	Canonical / Proposed
FEAT-EM-29 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION.FEAT-EM-29	Feature: Device Serial Tracking (p. 310)	Canonical / Proposed
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FEAT-EM-31 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-OA-M-METADATA-MANAGEMENT.FEAT-EM-31	Feature: Device Endpoint Registration (p. 318)	Canonical / Proposed
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FEAT-HSO-02 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-HSO-02	Feature: Operational Queue Progression (p. 518)	Canonical / Proposed
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FEAT-HSO-04 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-HSO-04	Feature: Clinical Handover Context Assembly (p. 528)	Canonical / Proposed
FEAT-HSO-05 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-HSO-05	Feature: Operating Theatre Case Progression (p. 541)	Canonical / Proposed
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FEAT-HSO-07 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-HSO-07	Feature: ED Bed & Bay Tracking (p. 551)	Canonical / Proposed
FEAT-HSO-08 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-HSO-08	Feature: ED Length of Stay Tracking (p. 552)	Canonical / Proposed

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FEAT-HSO-09 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-HSO-09	Feature: Outpatient Arrival Registration (p. 564)	Canonical / Proposed
FEAT-HSO-10 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-HSO-10	Feature: Post-Clinic Order Coordination (p. 565)	Canonical / Proposed
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FEAT-HSO-13 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION.FEAT-HSO-13	Feature: Bed Readiness Notification (p. 576)	Canonical / Proposed
FEAT-HSO-14 CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-HSO-14	Feature: Mobile Clinical Equipment Tracking (p. 313)	Canonical / Proposed
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FEAT-HSO-19 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-19	Feature: Work Item Instantiation (p. 610)	Canonical / Proposed
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FEAT-HSO-21 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-21	Feature: Work Dispatch Delivery (p. 612)	Canonical / Proposed
FEAT-HSO-22 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-22	Feature: Work Progress Oversight & Escalation (p. 613)	Canonical / Proposed
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FEAT-HSO-24 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION.FEAT-HSO-24	Feature: Transport Request Ingestion (p. 635)	Canonical / Proposed
FEAT-HSO-25 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION.FEAT-HSO-25	Feature: Transport Dispatch & Progress Tracking (p. 636)	Canonical / Proposed
FEAT-HSO-26 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.CT3-BEC-SPECIMEN-MOVEMENT-TRACEABILITY.FEAT-HSO-26	Feature: Pathology Specimen Transport Tracking (p. 643)	Canonical / Proposed
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FEAT-HSO-28 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-28	Feature: Discharge Readiness & Coordination Oversight (p. 649)	Canonical / Proposed
FEAT-HSO-29 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-29	Feature: Discharge Notification Dispatch (p. 650)	Canonical / Proposed
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FEAT-ISE-03 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-03	Feature: Durable Clinical Record Preservation (p. 661)	Canonical / Proposed

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FEAT-ISE-04 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-04	Feature: Direct Submission Ingestion & Delivery (p. 690)	Canonical / Proposed
FEAT-ISE-05 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-05	Feature: Clinical Record Retrieval Mediation (p. 691)	Canonical / Proposed
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FEAT-ISE-09 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.CT3-BEC-GOVERNED-HEALTH-INFORMATION-RETRIEVAL.FEAT-ISE-09	Feature: Access-Controlled Information Filtering (p. 705)	Canonical / Proposed
FEAT-ISE-10 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION.FEAT-ISE-10	Feature: Standards-Based Boundary Exchange (p. 892)	Canonical / Proposed
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FEAT-ISE-15 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-15	Feature: Bidirectional Terminology Mapping (p. 746)	Canonical / Proposed

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FEAT-ISE-16 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-16	Feature: Care-Team Clinical Collaboration Coordination (p. 759)	Canonical / Proposed
FEAT-ISE-17 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-17	Feature: In-Conversation LHR Query Resolution (p. 760)	Canonical / Proposed
FEAT-ISE-18 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-18	Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	Canonical / Proposed
FEAT-ISE-19 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.CT3-BEC-OPERATIONAL-COLLABORATION.FEAT-ISE-19	Feature: Operational Team Chat Enablement (p. 771)	Canonical / Proposed
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FEAT-ISE-21 CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-21	Feature: To Do Decision & Approval Coordination (p. 895)	Canonical / Proposed
FEAT-ISE-22 CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-22	Feature: Synthetic Task Orchestration (p. 896)	Canonical / Proposed
FEAT-ISE-23 CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-ISE-23	Feature: Workflow Timeout & Escalation Oversight (p. 897)	Canonical / Proposed
FEAT-ISE-24 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION.FEAT-ISE-24	Feature: Schedule Projection Integration (p. 781)	Canonical / Proposed
FEAT-ISE-25 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION.FEAT-ISE-25	Feature: Temporal Event Correlation (p. 782)	Canonical / Proposed
FEAT-ISE-26 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.CT3-BEC-CONTEXTUAL-INFORMATION-PRESENTATION.FEAT-ISE-26	Feature: Longitudinal Clinical & Operational Presentation (p. 789)	Canonical / Proposed
FEAT-ISE-27 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-ISE-27	Feature: Information Standards & Semantic Governance (p. 795)	Canonical / Proposed

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FEAT-ISE-28 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-UTILISATION-SUPPORT.FEAT-ISE-28	Feature: Semantic Conformance Verification (p. 801)	Canonical / Proposed
FEAT-SA-01 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE.FEAT-SA-01	Feature: Demographic Update Ingestion (p. 337)	Canonical / Proposed
FEAT-SA-02 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE.FEAT-SA-02	Feature: Demographic Change Distribution (p. 338)	Canonical / Proposed
FEAT-SA-03 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-03	Feature: Referral Document Ingestion (p. 346)	Canonical / Proposed
FEAT-SA-04 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-04	Feature: Referral Supporting Information (p. 346)	Canonical / Proposed
FEAT-SA-05 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-05	Feature: Referral Status & Outcome Tracking (p. 347)	Canonical / Proposed
FEAT-SA-06 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION.FEAT-SA-06	Feature: Appointment Notification Ingestion (p. 358)	Canonical / Proposed
FEAT-SA-07 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION.FEAT-SA-07	Feature: Appointment Status Synchronization (p. 359)	Canonical / Proposed
FEAT-SA-08 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-08	Feature: Encounter Context Creation (p. 373)	Canonical / Proposed
FEAT-SA-09 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-09	Feature: Encounter State Progression (p. 374)	Canonical / Proposed
FEAT-SA-10 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-10	Feature: Encounter Bed/Location Association (p. 375)	Canonical / Proposed
FEAT-SA-11 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-11	Feature: Order Request Ingestion (p. 386)	Canonical / Proposed

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FEAT-SA-12 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-12	Feature: Order Destination Resolution & Routing (p. 387)	Canonical / Proposed
FEAT-SA-13 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-13	Feature: Order Closed-Loop Progression Tracking (p. 388)	Canonical / Proposed
FEAT-SA-14 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-14	Feature: Order Cancellation & Modification Coordination (p. 389)	Canonical / Proposed
FEAT-SA-15 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-15	Feature: Diagnostic Request Correlation (p. 409)	Canonical / Proposed
FEAT-SA-16 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-16	Feature: Diagnostic Report Ingestion & Binding (p. 410)	Canonical / Proposed
FEAT-SA-17 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-17	Feature: Diagnostic Report Distribution (p. 411)	Canonical / Proposed
FEAT-SA-18 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-18	Feature: Medication Order Ingestion (p. 428)	Canonical / Proposed
FEAT-SA-19 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-19	Feature: Dispense Event Tracking (p. 429)	Canonical / Proposed
FEAT-SA-20 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-20	Feature: Medication Administration Record (MAR) Ingestion (p. 430)	Canonical / Proposed
FEAT-SA-21 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION.FEAT-SA-21	Feature: Procedure Booking Ingestion (p. 438)	Canonical / Proposed
FEAT-SA-22 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION.FEAT-SA-22	Feature: Procedural Documentation Ingestion (p. 439)	Canonical / Proposed
FEAT-SA-23 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-23	Feature: Clinical Document Ingestion (p. 447)	Canonical / Proposed

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FEAT-SA-24 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-24	Feature: Document Versioning & Supersession (p. 448)	Canonical / Proposed
FEAT-SA-25 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-25	Feature: Document Metadata Indexing (p. 449)	Canonical / Proposed
FEAT-SA-26 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION.FEAT-SA-26	Feature: Care Plan Ingestion & Distribution (p. 458)	Canonical / Proposed
FEAT-SA-27 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-27	Feature: Clinical Communication Distribution (p. 479)	Canonical / Proposed
FEAT-SA-28 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-28	Feature: Delivery Acknowledgement Tracking (p. 480)	Canonical / Proposed
FEAT-SA-29 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-29	Feature: Communication Audit Logging (p. 481)	Canonical / Proposed
FEAT-SD-01 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FEAT-SD-01	Feature: General Practice Shared Record Ingestion (p. 671)	Canonical / Proposed
FEAT-SD-02 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FEAT-SD-02	Feature: Primary Care Notification Dispatch (p. 671)	Canonical / Proposed
FEAT-SD-03 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-03	Feature: Acute Clinical State Event Capture (p. 366)	Canonical / Proposed
FEAT-SD-04 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-04	Feature: Acute Care Clinical Context Provision (p. 367)	Canonical / Proposed
FEAT-SD-05 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FEAT-SD-05	Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	Canonical / Proposed
FEAT-SD-06 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FEAT-SD-06	Feature: Emergency Care Cross-Facility Correlation (p. 556)	Canonical / Proposed
FEAT-SD-07 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FEAT-SD-07	Feature: Inpatient Rounding Context Provision (p. 531)	Canonical / Proposed

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FEAT-SD-08 CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FEAT-SD-08	Feature: Inpatient Clinical Progression Tracking (p. 532)	Canonical / Proposed
FEAT-SD-09 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-09	Feature: Laboratory & Imaging Result Distribution (p. 403)	Canonical / Proposed
FEAT-SD-10 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-10	Feature: Diagnostic History Consolidation (p. 404)	Canonical / Proposed
FEAT-SD-11 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-11	Feature: Medication History Reconciliation Ingestion (p. 421)	Canonical / Proposed
FEAT-SD-12 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-12	Feature: Medication Administration Tracking (p. 422)	Canonical / Proposed
FEAT-SD-13 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-13	Feature: Immunisation Event Submission (p. 331)	Canonical / Proposed
FEAT-SD-14 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-14	Feature: Screening Recall Notification Distribution (p. 332)	Canonical / Proposed
FEAT-SD-15 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-15	Feature: Community Encounter Ingestion (p. 462)	Canonical / Proposed
FEAT-SD-16 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-16	Feature: Community Care Plan Synchronization (p. 463)	Canonical / Proposed
FEAT-SD-17 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-17	Feature: Outreach Visit Context Provision (p. 468)	Canonical / Proposed
FEAT-SD-18 CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-18	Feature: Outreach Encounter Outcome Ingestion (p. 469)	Canonical / Proposed
FEAT-SD-19 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-19	Feature: Remote Telemetry Ingestion (p. 677)	Canonical / Proposed
FEAT-SD-20 CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-20	Feature: Virtual Care Session Context Binding (p. 678)	Canonical / Proposed
HCM-BC-access-control-assurance-reporting CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ASSURANCE-REPORTING	BC CT3: Access Control Assurance & Reporting (p. 147)	Canonical / Established
HCM-BC-access-control-collaboration CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-COLLABORATION	BC CT3: Access Control Collaboration (p. 147)	Canonical / Established

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HCM-BC-access-control-context-management CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-CONTEXT-MANAGEMENT	BC CT3: Access Control Context Management (p. 146)	Canonical / Established
HCM-BC-access-control-enforcement CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ENFORCEMENT	BC CT3: Access Control Enforcement (p. 148)	Canonical / Established
HCM-BC-assurance-approach-definition CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-APPROACH-DEFINITION	BC CT3: Assurance Approach Definition (p. 204)	Canonical / Established
HCM-BC-assurance-conclusion-determination CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-CONCLUSION-DETERMINATION	BC CT3: Assurance Conclusion Determination (p. 208)	Canonical / Established
HCM-BC-assurance-evaluation CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION	BC CT2: Assurance Evaluation (p. 206)	Canonical / Established
HCM-BC-assurance-evaluation-criteria-governance CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-EVALUATION-CRITERIA-GOVERNANCE	BC CT3: Assurance Evaluation Criteria Governance (p. 205)	Canonical / Established
HCM-BC-assurance-evidence-assessment CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-EVIDENCE-ASSESSMENT	BC CT3: Assurance Evidence Assessment (p. 207)	Canonical / Established
HCM-BC-authority-constraint-governance CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-AUTHORITY-CONSTRAINT-GOVERNANCE	BC CT3: Authority & Constraint Governance (p. 198)	Canonical / Established
HCM-BC-back-office-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-BACK-OFFICE-SERVICES	BC CT3: Back-Office Services (p. 189)	Canonical / Established
HCM-BC-care-access-coordination CT1-CARE-ACCESS-COORDINATION	BC CT1: Care Access & Coordination (p. 26)	Canonical / Established
HCM-BC-care-access-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT	BC CT2: Care Access Management (p. 27)	Canonical / Established
HCM-BC-care-appointment-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-APPOINTMENT-MANAGEMENT	BC CT3: Care Appointment Management (p. 32)	Canonical / Established
HCM-BC-care-appropriateness-review CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-APPROPRIATENESS-REVIEW	BC CT3: Care Appropriateness Review (p. 23)	Canonical / Established
HCM-BC-care-coordination-transition CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION	BC CT2: Care Coordination & Transition (p. 37)	Canonical / Established
HCM-BC-care-discharge-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-DISCHARGE-COORDINATION	BC CT3: Care Discharge Coordination (p. 40)	Canonical / Established
HCM-BC-care-effectiveness-review CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-EFFECTIVENESS-REVIEW	BC CT3: Care Effectiveness Review (p. 24)	Canonical / Established

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HCM-BC-care-eligibility-determination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-ELIGIBILITY-DETERMINATION	BC CT3: Care Eligibility Determination (p. 30)	Canonical / Established
HCM-BC-care-goal-definition CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING.CT3-BC-CARE-GOAL-DEFINITION	BC CT3: Care Goal Definition (p. 16)	Proposed / Proposed
HCM-BC-care-intake-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-INTAKE-MANAGEMENT	BC CT3: Care Intake Management (p. 28)	Canonical / Established
HCM-BC-care-place-allocation CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-ALLOCATION	BC CT3: Care-Place Allocation (p. 33)	Canonical / Established
HCM-BC-care-place-readiness-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-READINESS-MANAGEMENT	BC CT3: Care-Place Readiness Management (p. 34)	Canonical / Established
HCM-BC-care-referral-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-REFERRAL-MANAGEMENT	BC CT3: Care Referral Management (p. 29)	Canonical / Established
HCM-BC-care-resource-deployment CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-RESOURCE-DEPLOYMENT	BC CT3: Care Resource Deployment (p. 34)	Canonical / Established
HCM-BC-care-response-monitoring CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-CARE-RESPONSE-MONITORING	BC CT3: Care Response Monitoring (p. 21)	Canonical / Established
HCM-BC-care-response-planning CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING.CT3-BC-CARE-RESPONSE-PLANNING	BC CT3: Care Response Planning (p. 17)	Proposed / Proposed
HCM-BC-care-responsibility-transition CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-RESPONSIBILITY-TRANSITION	BC CT3: Care Responsibility Transition (p. 39)	Canonical / Established
HCM-BC-care-scheduling-capacity-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT	BC CT2: Care Scheduling & Capacity Management (p. 31)	Canonical / Established
HCM-BC-care-support-logistics-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT	BC CT2: Care Support Logistics Management (p. 41)	Canonical / Established
HCM-BC-care-support-work-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CARE-SUPPORT-WORK-COORDINATION	BC CT3: Care Support Work Coordination (p. 44)	Canonical / Established
HCM-BC-care-team-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-TEAM-COORDINATION	BC CT3: Care-Team Coordination (p. 38)	Canonical / Established
HCM-BC-classification-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-CLASSIFICATION-GOVERNANCE	BC CT3: Classification Governance (p. 132)	Canonical / Established

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HCM-BC-classification-utilisation-oversight CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-CLASSIFICATION-UTILISATION-OVERSIGHT	BC CT3: Classification Utilisation Oversight (p. 133)	Canonical / Established
HCM-BC-clinical-activity-charging-information-provision CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES.CT3-CLINICAL-ACTIVITY-CHARGING-INFORMATION-PROVISION	BC CT3: Clinical Activity Charging Information Provision (p. 187)	Canonical / Established
HCM-BC-clinical-determination CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION	BC CT2: Clinical Determination (p. 10)	Canonical / Established
HCM-BC-clinical-device-data-integration CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-DATA-INTEGRATION	BC CT3: Clinical Device Data Integration (p. 71)	Canonical / Established
HCM-BC-clinical-device-feature-release-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-FEATURE-RELEASE-MANAGEMENT	BC CT3: Clinical Device Feature & Release Management (p. 68)	Canonical / Established
HCM-BC-clinical-device-integration-provision CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-INTEGRATION-PROVISION	BC CT3: Clinical Device Integration Provision (p. 71)	Canonical / Established
HCM-BC-clinical-device-inventory-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-INVENTORY-MANAGEMENT	BC CT3: Clinical Device Inventory Management (p. 67)	Canonical / Established
HCM-BC-clinical-device-lifecycle-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LIFECYCLE-MANAGEMENT	BC CT3: Clinical Device Lifecycle Management (p. 72)	Canonical / Established
HCM-BC-clinical-device-loan-deployment-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LOAN-DEPLOYMENT-MANAGEMENT	BC CT3: Clinical Device Loan & Deployment Management (p. 69)	Canonical / Established
HCM-BC-clinical-device-maintenance-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-BC-CLINICAL-DEVICE-MAINTENANCE-MANAGEMENT	BC CT3: Clinical Device Maintenance Management (p. 73)	Candidate / Proposed
HCM-BC-clinical-device-utilisation-support CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-UTILISATION-SUPPORT	BC CT3: Clinical Device Utilisation Support (p. 70)	Canonical / Established
HCM-BC-clinical-examination CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-CLINICAL-EXAMINATION	BC CT3: Clinical Examination (p. 8)	Canonical / Established

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HCM-BC-clinical-finding-interpretation CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION.CT3-BC-CLINICAL-FINDING-INTERPRETATION	BC CT3: Clinical Finding Interpretation (p. 11)	Proposed / Proposed
HCM-BC-clinical-intervention CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-INTERVENTION	BC CT2: Clinical Intervention (p. 18)	Canonical / Established
HCM-BC-clinical-knowledge-concept-association CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONCEPT-ASSOCIATION	BC CT3: Clinical Knowledge Concept Association (p. 116)	Canonical / Established
HCM-BC-clinical-knowledge-context-extraction CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-EXTRACTION	BC CT3: Clinical Knowledge Context Extraction (p. 115)	Canonical / Established
HCM-BC-clinical-knowledge-context-identification CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-IDENTIFICATION	BC CT3: Clinical Knowledge Context Identification (p. 114)	Canonical / Established
HCM-BC-clinical-knowledge-information-formulation CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-INFORMATION-FORMULATION	BC CT3: Clinical Knowledge Information Formulation (p. 116)	Canonical / Established
HCM-BC-clinical-knowledge-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT	BC CT2: Clinical Knowledge Management (p. 112)	Canonical / Established
HCM-BC-clinical-knowledge-publication-sharing CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-PUBLICATION-SHARING	BC CT3: Clinical Knowledge Publication & Sharing (p. 118)	Canonical / Established
HCM-BC-clinical-knowledge-review-approval-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-REVIEW-APPROVAL-MANAGEMENT	BC CT3: Clinical Knowledge Review & Approval Management (p. 117)	Canonical / Established
HCM-BC-clinical-material-logistics-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CLINICAL-MATERIAL-LOGISTICS-COORDINATION	BC CT3: Clinical Material Logistics Coordination (p. 43)	Canonical / Established
HCM-BC-clinical-quality-criteria-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-CRITERIA-MANAGEMENT	BC CT3: Clinical Quality Criteria Management (p. 81)	Canonical / Established
HCM-BC-clinical-quality-evidence-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-EVIDENCE-MANAGEMENT	BC CT3: Clinical Quality Evidence Management (p. 81)	Canonical / Established
HCM-BC-clinical-quality-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT	BC CT2: Clinical Quality Management (p. 79)	Canonical / Established

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HCM-BC-clinical-quality-reporting CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REPORTING	BC CT3: Clinical Quality Reporting (p. 83)	Canonical / Established
HCM-BC-clinical-quality-review-assessment-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REVIEW-ASSESSMENT-MANAGEMENT	BC CT3: Clinical Quality Review & Assessment Management (p. 82)	Canonical / Established
HCM-BC-clinical-quality-safety-improvement CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT	BC CT1: Clinical Quality, Safety & Improvement (p. 75)	Canonical / Established
HCM-BC-clinical-risk-identification CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-IDENTIFICATION	BC CT3: Clinical Risk Identification (p. 77)	Canonical / Established
HCM-BC-clinical-risk-mitigation-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-MITIGATION-MANAGEMENT	BC CT3: Clinical Risk Mitigation Management (p. 78)	Canonical / Established
HCM-BC-clinical-risk-response-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-RESPONSE-MANAGEMENT	BC CT3: Clinical Risk Response Management (p. 79)	Canonical / Established
HCM-BC-clinical-safety-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT	BC CT2: Clinical Safety Management (p. 76)	Canonical / Established
HCM-BC-clinical-semantics-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE	BC CT2: Clinical Semantics Governance (p. 128)	Canonical / Established
HCM-BC-clinical-technology-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT	BC CT2: Clinical Technology Management (p. 66)	Canonical / Established
HCM-BC-communications-publicity CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-COMMUNICATIONS-PUBLICITY	BC CT3: Communications & Publicity (p. 192)	Canonical / Established
HCM-BC-community-health-wellbeing CT1-COMMUNITY-HEALTH-WELLBEING	BC CT1: Community Health & Wellbeing (p. 85)	Canonical / Established
HCM-BC-compliance-assessment-definition CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-COMPLIANCE-ASSESSMENT-DEFINITION	BC CT3: Compliance Assessment Definition (p. 200)	Canonical / Established
HCM-BC-compliance-governance CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE	BC CT2: Compliance Governance (p. 199)	Canonical / Established
HCM-BC-connected-health-services CT1-CONNECTED-HEALTH-SERVICES	BC CT1: Connected Health Services (p. 135)	Canonical / Established
HCM-BC-corporate-resources-enterprise-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES	BC CT1: Corporate Resources & Enterprise Services (p. 185)	Canonical / Established
HCM-BC-corporate-support-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES	BC CT2: Corporate Support Services (p. 188)	Canonical / Established
HCM-BC-diagnosis-problem-determination CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION.CT3-BC-DIAGNOSIS-PROBLEM-DETERMINATION	BC CT3: Diagnosis & Problem Determination (p. 12)	Proposed / Proposed

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HCM-BC-diagnostic-data-management CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-DATA-MANAGEMENT	BC CT3: Diagnostic Data Management (p. 55)	Canonical / Established
HCM-BC-diagnostic-interpretation CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-INTERPRETATION	BC CT3: Diagnostic Interpretation (p. 56)	Canonical / Established
HCM-BC-diagnostic-report-distribution CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-DISTRIBUTION	BC CT3: Diagnostic Report Distribution (p. 57)	Canonical / Established
HCM-BC-diagnostic-report-management CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-MANAGEMENT	BC CT3: Diagnostic Report Management (p. 56)	Canonical / Established
HCM-BC-diagnostic-service-appointment-management CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SERVICE-APPOINTMENT-MANAGEMENT	BC CT3: Diagnostic Service Appointment Management (p. 52)	Canonical / Established
HCM-BC-diagnostic-service-delivery CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SERVICE-DELIVERY	BC CT3: Diagnostic Service Delivery (p. 53)	Canonical / Established
HCM-BC-diagnostic-specimen-management CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SPECIMEN-MANAGEMENT	BC CT3: Diagnostic Specimen Management (p. 54)	Canonical / Established
HCM-BC-diagnostic-support-provision CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION	BC CT2: Diagnostic Support Provision (p. 51)	Canonical / Established
HCM-BC-diagnostic-therapeutic-clinical-support-services CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES	BC CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 50)	Canonical / Established
HCM-BC-digital-accountability-trust CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST	BC CT2: Digital Accountability & Trust (p. 155)	Canonical / Established
HCM-BC-digital-communication-channel CT1-CONNECTED-HEALTH-SERVICES.CT2-DIGITAL-COMMUNICATION-CHANNEL	BC CT2: Digital Communication Channel (p. 138)	Canonical / Established
HCM-BC-digital-identity-access-governance CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE	BC CT2: Digital Identity & Access Governance (p. 141)	Canonical / Established
HCM-BC-facilities-management CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-FACILITIES-MANAGEMENT	BC CT3: Facilities Management (p. 190)	Canonical / Established
HCM-BC-financial-commercial-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES	BC CT2: Financial & Commercial Services (p. 186)	Canonical / Established
HCM-BC-governance-framework-management CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT	BC CT2: Governance Framework Management (p. 196)	Canonical / Established

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HCM-BC-governance-reporting-checkpoint-definition CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-GOVERNANCE-REPORTING-CHECKPOINT-DEFINITION	BC CT3: Governance Reporting & Checkpoint Definition (p. 201)	Canonical / Established
HCM-BC-governed-activity-attribution CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-GOVERNED-ACTIVITY-ATTRIBUTION	BC CT3: Governed Activity Attribution (p. 156)	Canonical / Established
HCM-BC-governed-research-information-provision CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT.CT3-GOVERNED-RESEARCH-INFORMATION-PROVISION	BC CT3: Governed Research Information Provision (p. 161)	Canonical / Established
HCM-BC-health-history-assessment CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-HEALTH-HISTORY-ASSESSMENT	BC CT3: Health History Assessment (p. 7)	Canonical / Established
HCM-BC-health-information-access-monitoring CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-ACCESS-MONITORING	BC CT3: Health Information Access Monitoring (p. 152)	Canonical / Established
HCM-BC-health-information-catalogue-and-index-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-CATALOGUE-AND-INDEX-MANAGEMENT	BC CT3: Health Information Catalogue and Index Management (p. 109)	Canonical / Established
HCM-BC-health-information-classification-assurance CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-CLASSIFICATION-ASSURANCE	BC CT3: Health Information Classification Assurance (p. 154)	Canonical / Established
HCM-BC-health-information-compartment-assurance CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-COMPARTMENT-ASSURANCE	BC CT3: Health Information Compartment Assurance (p. 153)	Canonical / Established
HCM-BC-health-information-compartmentalisation CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-COMPARTMENTALISATION	BC CT3: Health Information Compartmentalisation (p. 110)	Canonical / Established
HCM-BC-health-information-data-correlation CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-CORRELATION	BC CT3: Health Information Data Correlation (p. 111)	Canonical / Established
HCM-BC-health-information-data-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-MANAGEMENT	BC CT3: Health Information Data Management (p. 106)	Canonical / Established
HCM-BC-health-information-data-retrieval CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-RETRIEVAL	BC CT3: Health Information Data Retrieval (p. 112)	Canonical / Established
HCM-BC-health-information-exchange CT1-CONNECTED-HEALTH-SERVICES.CT2-HEALTH-INFORMATION-EXCHANGE	BC CT2: Health Information Exchange (p. 136)	Canonical / Established

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HCM-BC-health-information-knowledge-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT	BC CT1: Health Information & Knowledge Management (p. 104)	Canonical / Established
HCM-BC-health-information-lifecycle-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-LIFECYCLE-MANAGEMENT	BC CT3: Health Information Lifecycle Management (p. 108)	Canonical / Established
HCM-BC-health-information-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT	BC CT2: Health Information Management (p. 105)	Canonical / Established
HCM-BC-health-information-protection CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION	BC CT2: Health Information Protection (p. 149)	Canonical / Established
HCM-BC-health-information-safeguarding CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-SAFEGUARDING	BC CT3: Health Information Safeguarding (p. 151)	Canonical / Established
HCM-BC-health-information-sharing-programme-management CT1-CONNECTED-HEALTH-SERVICES.CT2-HEALTH-INFORMATION-EXCHANGE.CT3-BC-HEALTH-INFORMATION-SHARING-PROGRAMME-MANAGEMENT	BC CT3: Health Information Sharing Programme Management (p. 137)	Candidate / Proposed
HCM-BC-health-information-standards-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE	BC CT2: Health Information Standards Governance (p. 121)	Canonical / Established
HCM-BC-health-information-structure-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-STRUCTURE-MANAGEMENT	BC CT3: Health Information Structure Management (p. 107)	Canonical / Established
HCM-BC-health-innovation CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION	BC CT2: Health Innovation (p. 162)	Canonical / Established
HCM-BC-health-innovation-evaluation CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION.CT3-HEALTH-INNOVATION-EVALUATION	BC CT3: Health Innovation Evaluation (p. 163)	Canonical / Established
HCM-BC-health-privacy-safeguard-management CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-PRIVACY-SAFEGUARD-MANAGEMENT	BC CT3: Health Privacy Safeguard Management (p. 150)	Canonical / Established
HCM-BC-health-products-clinical-technology CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY	BC CT1: Health Products & Clinical Technology (p. 65)	Canonical / Established
HCM-BC-health-research-innovation CT1-HEALTH-RESEARCH-INNOVATION	BC CT1: Health Research & Innovation (p. 159)	Canonical / Established
HCM-BC-health-rights-advocacy-participation CT1-HEALTH-RIGHTS-ADVOCACY-PARTICIPATION	BC CT1: Health Rights, Advocacy & Participation (p. 48)	Canonical / Established
HCM-BC-health-risk-prognostic-determination CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION.CT3-BC-HEALTH-RISK-PROGNOSTIC-DETERMINATION	BC CT3: Health Risk & Prognostic Determination (p. 13)	Proposed / Proposed

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HCM-BC-health-service-alignment CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT	BC CT2: Health Service Alignment (p. 170)	Canonical / Established
HCM-BC-health-service-assurance CT1-HEALTH-SERVICE-ASSURANCE	BC CT1: Health Service Assurance (p. 202)	Canonical / Established
HCM-BC-health-service-assurance-specification CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION	BC CT2: Health Service Assurance Specification (p. 203)	Canonical / Established
HCM-BC-health-service-capability-identification CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-CAPABILITY-IDENTIFICATION	BC CT3: Health Service Capability Identification (p. 168)	Canonical / Established
HCM-BC-health-service-capacity-planning CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-CAPACITY-PLANNING	BC CT3: Health Service Capacity Planning (p. 102)	Canonical / Established
HCM-BC-health-service-co-dependency-management CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-CO-DEPENDENCY-MANAGEMENT	BC CT3: Health Service Co-Dependency Management (p. 172)	Canonical / Established
HCM-BC-health-service-continuous-improvement CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-CONTINUOUS-IMPROVEMENT	BC CT3: Health Service Continuous Improvement (p. 170)	Canonical / Established
HCM-BC-health-service-demand-assessment CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-DEMAND-ASSESSMENT	BC CT3: Health Service Demand Assessment (p. 101)	Canonical / Established
HCM-BC-health-service-direction-stewardship CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP	BC CT1: Health Service Direction & Stewardship (p. 165)	Canonical / Established
HCM-BC-health-service-distribution-planning CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-DISTRIBUTION-PLANNING	BC CT3: Health Service Distribution Planning (p. 103)	Canonical / Established
HCM-BC-health-service-financial-management CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-FINANCIAL-MANAGEMENT	BC CT3: Health Service Financial Management (p. 173)	Canonical / Established
HCM-BC-health-service-governance CT1-HEALTH-SERVICE-GOVERNANCE	BC CT1: Health Service Governance (p. 195)	Canonical / Established
HCM-BC-health-service-implementation-planning CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-IMPLEMENTATION-PLANNING	BC CT3: Health Service Implementation Planning (p. 174)	Canonical / Established
HCM-BC-health-service-implementation-support CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-IMPLEMENTATION-SUPPORT	BC CT3: Health Service Implementation Support (p. 169)	Canonical / Established
HCM-BC-health-service-modelling CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING	BC CT2: Health Service Modelling (p. 166)	Canonical / Established

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HCM-BC-health-service-need-identification CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP. CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-NEED-IDENTIFICATION	BC CT3: Health Service Need Identification (p. 167)	Canonical / Established
HCM-BC-health-service-operational-readiness CT1-CARE-ACCESS-COORDINATION.CT2-BC-SERVICE-DELIVERY-PREPAREDNESS-MANAGEMENT.CT3-BC-HEALTH-SERVICE-OPERATIONAL-READINESS	BC CT3: Health Service Operational Readiness (p. 46)	Candidate / Proposed
HCM-BC-health-service-resource-identification CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP. CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-RESOURCE-IDENTIFICATION	BC CT3: Health Service Resource Identification (p. 172)	Canonical / Established
HCM-BC-health-state-monitoring CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-HEALTH-STATE-MONITORING	BC CT3: Health State Monitoring (p. 20)	Canonical / Established
HCM-BC-health-status-measurement CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-HEALTH-STATUS-MEASUREMENT	BC CT3: Health Status Measurement (p. 9)	Canonical / Established
HCM-BC-health-system-planning CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING	BC CT2: Health-System Planning (p. 99)	Canonical / Established
HCM-BC-health-workforce-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT	BC CT2: Health Workforce Management (p. 177)	Canonical / Established
HCM-BC-healthcare-location-definition-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-LOCATION-DEFINITION-MANAGEMENT	BC CT3: Healthcare Location Definition Management (p. 183)	Canonical / Established
HCM-BC-healthcare-organisation-identity-structure-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-ORGANISATION-IDENTITY-STRUCTURE-MANAGEMENT	BC CT3: Healthcare Organisation Identity & Structure Management (p. 182)	Canonical / Established
HCM-BC-healthcare-organisation-service-structure-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT	BC CT2: Healthcare Organisation & Service Structure Management (p. 181)	Canonical / Established
HCM-BC-healthcare-service-offering-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-SERVICE-OFFERING-MANAGEMENT	BC CT3: Healthcare Service Offering Management (p. 184)	Canonical / Established
HCM-BC-identity-assurance CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-ASSURANCE	BC CT3: Identity Assurance (p. 142)	Canonical / Established
HCM-BC-identity-identification-authentication CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-IDENTIFICATION-AUTHENTICATION	BC CT3: Identity Identification & Authentication (p. 145)	Canonical / Established

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HCM-BC-identity-modelling-interoperability CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-MODELLING-INTEROPERABILITY	BC CT3: Identity Modelling & Interoperability (p. 143)	Canonical / Established
HCM-BC-identity-qualification-control CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-QUALIFICATION-CONTROL	BC CT3: Identity Qualification Control (p. 144)	Canonical / Established
HCM-BC-individual-care-delivery CT1-INDIVIDUAL-CARE-DELIVERY	BC CT1: Individual Care Delivery (p. 5)	Canonical / Established
HCM-BC-individual-care-planning CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING	BC CT2: Individual Care Planning (p. 14)	Canonical / Established
HCM-BC-individual-care-review CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW	BC CT2: Individual Care Review (p. 22)	Canonical / Established
HCM-BC-individual-health-assessment CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT	BC CT2: Individual Health Assessment (p. 6)	Canonical / Established
HCM-BC-individual-health-monitoring CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING	BC CT2: Individual Health Monitoring (p. 19)	Canonical / Established
HCM-BC-information-model-adoption CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-ADOPTION	BC CT3: Information Model Adoption (p. 123)	Canonical / Established
HCM-BC-information-model-compliance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-COMPLIANCE	BC CT3: Information Model Compliance (p. 125)	Canonical / Established
HCM-BC-information-model-discovery CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-DISCOVERY	BC CT3: Information Model Discovery (p. 122)	Canonical / Established
HCM-BC-information-model-education CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-EDUCATION	BC CT3: Information Model Education (p. 124)	Canonical / Established
HCM-BC-information-model-governance-oversight CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-GOVERNANCE-OVERSIGHT	BC CT3: Information Model Governance Oversight (p. 125)	Canonical / Established
HCM-BC-it-infrastructure-management CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-IT-INFRASTRUCTURE-MANAGEMENT	BC CT3: IT Infrastructure Management (p. 191)	Canonical / Established
HCM-BC-medication-advice CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-MEDICATION-ADVICE	BC CT3: Medication Advice (p. 61)	Canonical / Established
HCM-BC-medication-compounding-preparation CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-BC-MEDICATION-COMPOUNDING-PREPARATION	BC CT3: Medication Compounding & Preparation (p. 63)	Proposed / Proposed

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HCM-BC-medication-dose-administration-aid-provision CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-MEDICATION-DOSE-ADMINISTRATION-AID-PROVISION	BC CT3: Medication Dose Administration Aid Provision (p. 62)	Canonical / Established
HCM-BC-medication-pharmacy-services CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES	BC CT2: Medication & Pharmacy Services (p. 58)	Canonical / Established
HCM-BC-non-prescription-medication-provision CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-NON-PRESCRIPTION-MEDICATION-PROVISION	BC CT3: Non-Prescription Medication Provision (p. 60)	Canonical / Established
HCM-BC-notifiable-health-event-management CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-NOTIFIABLE-HEALTH-EVENT-MANAGEMENT	BC CT3: Notifiable Health Event Management (p. 98)	Canonical / Established
HCM-BC-operational-care-capacity-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-OPERATIONAL-CARE-CAPACITY-MANAGEMENT	BC CT3: Operational Care Capacity Management (p. 36)	Canonical / Established
HCM-BC-operational-care-flow-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-OPERATIONAL-CARE-FLOW-COORDINATION	BC CT3: Operational Care Flow Coordination (p. 35)	Canonical / Established
HCM-BC-operational-oversight CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-OPERATIONAL-OVERSIGHT	BC CT3: Operational Oversight (p. 193)	Canonical / Established
HCM-BC-patient-movement-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-PATIENT-MOVEMENT-COORDINATION	BC CT3: Patient Movement Coordination (p. 42)	Canonical / Established
HCM-BC-people-management CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-PEOPLE-MANAGEMENT	BC CT3: People Management (p. 192)	Canonical / Established
HCM-BC-policy-obligation-governance CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-POLICY-OBLIGATION-GOVERNANCE	BC CT3: Policy / Obligation Governance (p. 197)	Canonical / Established
HCM-BC-population-health-event-identification CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-EVENT-IDENTIFICATION	BC CT3: Population Health Event Identification (p. 97)	Canonical / Established
HCM-BC-population-health-health-system-planning CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING	BC CT1: Population Health & Health-System Planning (p. 93)	Canonical / Established
HCM-BC-population-health-longitudinal-assessment CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-LONGITUDINAL-ASSESSMENT	BC CT3: Population Health Longitudinal Assessment (p. 95)	Canonical / Established

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HCM-BC-population-health-reporting CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-REPORTING	BC CT3: Population Health Reporting (p. 99)	Canonical / Established
HCM-BC-population-health-surveillance CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE	BC CT2: Population Health Surveillance (p. 94)	Canonical / Established
HCM-BC-population-health-surveillance-management CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-SURVEILLANCE-MANAGEMENT	BC CT3: Population Health Surveillance Management (p. 96)	Canonical / Established
HCM-BC-practitioner-engagement-privilege-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-ENGAGEMENT-PRIVILEGE-MANAGEMENT	BC CT3: Practitioner Engagement & Privilege Management (p. 179)	Canonical / Established
HCM-BC-practitioner-qualification-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-QUALIFICATION-MANAGEMENT	BC CT3: Practitioner Qualification Management (p. 178)	Canonical / Established
HCM-BC-prescription-medication-dispensing CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-PRESCRIPTION-MEDICATION-DISPENSING	BC CT3: Prescription Medication Dispensing (p. 60)	Canonical / Established
HCM-BC-preventative-health CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH	BC CT2: Preventative Health (p. 86)	Canonical / Established
HCM-BC-preventative-health-cohort-management CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-COHORT-MANAGEMENT	BC CT3: Preventative Health Cohort Management (p. 91)	Canonical / Established
HCM-BC-preventative-health-education-services CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-EDUCATION-SERVICES	BC CT3: Preventative Health Education Services (p. 89)	Canonical / Established
HCM-BC-preventative-health-problem-identification CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROBLEM-IDENTIFICATION	BC CT3: Preventative Health Problem Identification (p. 87)	Canonical / Established
HCM-BC-preventative-health-programme-management CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MANAGEMENT	BC CT3: Preventative Health Programme Management (p. 88)	Canonical / Established
HCM-BC-preventative-health-programme-monitoring CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MONITORING	BC CT3: Preventative Health Programme Monitoring (p. 90)	Canonical / Established
HCM-BC-preventative-health-reporting CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-REPORTING	BC CT3: Preventative Health Reporting (p. 90)	Canonical / Established

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HCM-BC-research-information-support CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT	BC CT2: Research Information Support (p. 160)	Canonical / Established
HCM-BC-security-information-use-accountability CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-SECURITY-INFORMATION-USE-ACCOUNTABILITY	BC CT3: Security & Information-Use Accountability (p. 157)	Canonical / Established
HCM-BC-security-privacy-digital-trust CT1-SECURITY-PRIVACY-DIGITAL-TRUST	BC CT1: Security, Privacy & Digital Trust (p. 140)	Canonical / Established
HCM-BC-semantic-mapping-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-SEMANTIC-MAPPING-GOVERNANCE	BC CT3: Semantic Mapping Governance (p. 133)	Canonical / Established
HCM-BC-service-delivery-preparedness-management CT1-CARE-ACCESS-COORDINATION.CT2-BC-SERVICE-DELIVERY-PREPAREDNESS-MANAGEMENT	BC CT2: Service Delivery Preparedness Management (p. 44)	Proposed / Proposed
HCM-BC-standards-adoption CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-STANDARDS-ADOPTION	BC CT3: Standards Adoption (p. 126)	Canonical / Established
HCM-BC-standards-impact-identification CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-STANDARDS-IMPACT-IDENTIFICATION	BC CT3: Standards Impact Identification (p. 127)	Canonical / Established
HCM-BC-standards-semantics-reference-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE	BC CT1: Standards, Semantics & Reference Governance (p. 120)	Canonical / Established
HCM-BC-terminology-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-GOVERNANCE	BC CT3: Terminology Governance (p. 129)	Canonical / Established
HCM-BC-terminology-localisation CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-LOCALISATION	BC CT3: Terminology Localisation (p. 131)	Canonical / Established
HCM-BC-terminology-utilisation-oversight CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-UTILISATION-OVERSIGHT	BC CT3: Terminology Utilisation Oversight (p. 130)	Canonical / Established
HCM-BC-workforce-organisational-capability CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY	BC CT1: Workforce & Organisational Capability (p. 176)	Canonical / Established
HCM-BC-workforce-service-coverage-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-WORKFORCE-SERVICE-COVERAGE-MANAGEMENT	BC CT3: Workforce Service Coverage Management (p. 180)	Canonical / Established
HCM-BEC-acute-care CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	Canonical / Proposed
HCM-BEC-allied-health CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-ALLIED-HEALTH	BEC CT2: Allied Health (p. 504)	Canonical / Proposed

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HCM-BEC-ambulatory-care CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-AMBULATORY-CARE	BEC CT2: Ambulatory Care (p. 498)	Canonical / Proposed
HCM-BEC-associated-clinical-binary-content-enablement CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT	BEC CT3: Linked Information Curation (p. 663)	Proposed / Proposed
HCM-BEC-assurance-criteria-management CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-CRITERIA-MANAGEMENT	BEC CT3: Assurance Criteria Administration (p. 828)	Canonical / Proposed
HCM-BEC-assurance-design CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN	BEC CT3: Assurance Design (p. 824)	Canonical / Proposed
HCM-BEC-assurance-specification-enablement CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT	BEC CT2: Assurance Specification (p. 822)	Proposed / Proposed
HCM-BEC-authorised-research-information-provision CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION	BEC CT2: Research Information Curation (p. 812)	Proposed / Proposed
HCM-BEC-bed-care-place-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT	BEC CT2: Bed & Care-Place Management (p. 569)	Canonical / Proposed
HCM-BEC-calendar-management CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT	BEC CT2: Calendar Management (p. 779)	Canonical / Proposed
HCM-BEC-care-access-transaction-enablement CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT	BEC CT2: Care Access Transaction Enablement (p. 937)	Proposed / Proposed
HCM-BEC-care-administration-information-enablement CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT	BEC CT1: Care Administration Enablement (p. 326)	Proposed / Proposed
HCM-BEC-care-context-integration-enablement CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT	BEC CT1: Care Context Integration Enablement (p. 923)	Proposed / Proposed
HCM-BEC-care-documentation-communication-enablement CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT	BEC CT2: Care Documentation & Communication Enablement (p. 940)	Proposed / Proposed
HCM-BEC-care-financial-information-enablement CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FINANCIAL-INFORMATION-ENABLEMENT	BEC CT2: Care Financial Information Enablement (p. 941)	Proposed / Proposed

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HCM-BEC-care-fulfilment-administration-enablement CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT	BEC CT2: Care Fulfilment Administration Enablement (p. 939)	Proposed / Proposed
HCM-BEC-care-movement-transition-coordination CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION	BEC CT2: Care Movement & Transition Coordination (p. 950)	Proposed / Proposed
HCM-BEC-care-operations-coordination-enablement CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT	BEC CT1: Health Service Operations Enablement (p. 512)	Proposed / Proposed
HCM-BEC-care-place-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-CARE-PLACE-DEFINITION	BEC CT3: Care-Place Definition Administration (p. 281)	Proposed / Proposed
HCM-BEC-care-plan-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION	BEC CT2: Care Plan Administration (p. 455)	Canonical / Proposed
HCM-BEC-care-resource-capacity-coordination CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION	BEC CT2: Care Resource & Capacity Coordination (p. 947)	Proposed / Proposed
HCM-BEC-care-setting-operations-enablement CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT	BEC CT2: Care Setting Operations Enablement (p. 946)	Proposed / Proposed
HCM-BEC-client-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION	BEC CT2: Client Administration (p. 212)	Canonical / Proposed
HCM-BEC-client-centred-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION	BEC CT2: Client-Centred Administration (p. 328)	Canonical / Proposed
HCM-BEC-client-privacy CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY	BEC CT3: Client Privacy Guardrail Management (p. 237)	Canonical / Proposed
HCM-BEC-client-relationship CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP	BEC CT3: Client Relationship Administration (p. 232)	Canonical / Proposed
HCM-BEC-clinic-practice-operations CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS	BEC CT2: Clinic & Practice Operations (p. 514)	Canonical / Proposed
HCM-BEC-clinical-assessment CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-CLINICAL-ASSESSMENT	BEC CT2: Clinical Assessment (p. 492)	Canonical / Proposed
HCM-BEC-clinical-charging-claims-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-CHARGING-CLAIMS-ADMINISTRATION	BEC CT2: Clinical Charging & Claims Administration (p. 489)	Canonical / Proposed

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HCM-BEC-clinical-collaboration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION	BEC CT2: Clinical Collaboration (p. 756)	Canonical / Proposed
HCM-BEC-clinical-communication-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION	BEC CT2: Clinical Communication Administration (p. 476)	Canonical / Proposed
HCM-BEC-clinical-competency-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-COMPETENCY-MANAGEMENT	BEC CT2: Clinical Competency Administration (p. 619)	Canonical / Proposed
HCM-BEC-clinical-device-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION	BEC CT2: Clinical Device Administration (p. 305)	Canonical / Proposed
HCM-BEC-clinical-information-stewardship-enablement CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT	BEC CT2: Clinical Information Stewardship Enablement (p. 952)	Proposed / Proposed
HCM-BEC-clinical-knowledge-services CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES	BEC CT2: Clinical Knowledge Services (p. 742)	Canonical / Proposed
HCM-BEC-clinical-logistics-coordination CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION	BEC CT2: Clinical Logistics Coordination (p. 639)	Canonical / Proposed
HCM-BEC-clinical-qualification-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT	BEC CT2: Clinical Qualification Administration (p. 621)	Canonical / Proposed
HCM-BEC-clinical-record-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION	BEC CT2: Clinical Record Administration (p. 443)	Canonical / Proposed
HCM-BEC-clinical-resource-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT	BEC CT3: Clinical Resource Management (p. 889)	Canonical / Proposed
HCM-BEC-clinical-rostering CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-ROSTERING	BEC CT2: Clinical Rostering (p. 589)	Canonical / Proposed
HCM-BEC-clinical-supervision CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-SUPERVISION	BEC CT2: Clinical Supervision (p. 630)	Canonical / Proposed
HCM-BEC-clinical-training-education CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-TRAINING-EDUCATION	BEC CT2: Clinical Training & Education (p. 618)	Canonical / Proposed
HCM-BEC-clinical-workforce-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-WORKFORCE-MANAGEMENT	BEC CT2: Clinical Workforce Capability Monitoring (p. 587)	Canonical / Proposed
HCM-BEC-collaborative-interaction-enablement CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT	BEC CT2: Collaborative Interaction Enablement (p. 956)	Proposed / Proposed

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HCM-BEC-community-care CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE	BEC CT3: Community Care Coordination (p. 460)	Canonical / Proposed
HCM-BEC-credential-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CREDENTIAL-MANAGEMENT	BEC CT3: Credential Management (p. 899)	Canonical / Proposed
HCM-BEC-device-association CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-ASSOCIATION	BEC CT3: Device Association (p. 935)	Proposed / Proposed
HCM-BEC-device-connectivity CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-CONNECTIVITY	BEC CT3: Device Connectivity (p. 936)	Proposed / Proposed
HCM-BEC-device-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION	BEC CT3: Clinical Device Profile Administration (p. 307)	Proposed / Proposed
HCM-BEC-device-instance CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-INSTANCE	BEC CT3: Device Instance (p. 934)	Proposed / Proposed
HCM-BEC-diagnostic-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION	BEC CT2: Diagnostic Administration (p. 399)	Canonical / Proposed
HCM-BEC-diagnostic-services CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES	BEC CT3: Diagnostic Service Integration (p. 401)	Canonical / Proposed
HCM-BEC-discharge-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT	BEC CT2: Discharge Management (p. 645)	Canonical / Proposed
HCM-BEC-emergency-care CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE	BEC CT3: Emergency Care Progression (p. 554)	Canonical / Proposed
HCM-BEC-emergency-department-operations CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS	BEC CT2: Emergency Department Operations (p. 548)	Canonical / Proposed
HCM-BEC-entity-specific-operational-coordination CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION	BEC CT3: Entity-Specific Operational Coordination (p. 963)	Proposed / Proposed
HCM-BEC-episode-encounter-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION	BEC CT2: Episode & Encounter Administration (p. 363)	Canonical / Proposed
HCM-BEC-facility-episodic-care-integration CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION	BEC CT2: Facility & Episodic Care Integration (p. 943)	Proposed / Proposed

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HCM-BEC-funding-coverage-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-FUNDING-COVERAGE-ADMINISTRATION	BEC CT2: Funding & Coverage Administration (p. 487)	Canonical / Proposed
HCM-BEC-general-collaboration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION	BEC CT2: General Collaboration (p. 768)	Canonical / Proposed
HCM-BEC-governance-requirement-publication-support CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-GOVERNANCE-REQUIREMENT-PUBLICATION-SUPPORT	BEC CT3: Governance Requirement Publication Support (p. 962)	Proposed / Proposed
HCM-BEC-governed-assurance CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE	BEC CT3: Governed Assurance (p. 898)	Canonical / Proposed
HCM-BEC-governed-health-information-enablement CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT	BEC CT1: Shared Information & Coordination Enablement (p. 654)	Proposed / Proposed
HCM-BEC-governed-operational-activity-design CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN	BEC CT3: Governed Operational Activity Design (p. 964)	Proposed / Proposed
HCM-BEC-governed-reference-dataset-administration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION	BEC CT3: Governed Reference Dataset Administration (p. 748)	Proposed / Proposed
HCM-BEC-health-information-access CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS	BEC CT2: Health Information Access (p. 701)	Canonical / Proposed
HCM-BEC-health-information-communication CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION	BEC CT3: Health Information Communication (p. 890)	Canonical / Proposed
HCM-BEC-health-information-connectivity-enablement CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT	BEC CT2: Health Information Connectivity Enablement (p. 953)	Proposed / Proposed
HCM-BEC-health-information-control CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL	BEC CT2: Health Information Control (p. 715)	Canonical / Proposed
HCM-BEC-health-information-exchange CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT	BEC CT2: Health Information Exchange Enablement (p. 686)	Canonical / Proposed
HCM-BEC-health-product-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION	BEC CT2: Health Materiel Administration (p. 300)	Canonical / Proposed

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HCM-BEC-health-product-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION.CT3-BEC-HEALTH-PRODUCT-DEFINITION	BEC CT3: Health Materiel Profile Administration (p. 301)	Proposed / Proposed
HCM-BEC-health-service-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION	BEC CT2: Health Service Administration (p. 287)	Canonical / Proposed
HCM-BEC-healthcare-entity-information-enablement CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT	BEC CT1: Entity Information Enablement (p. 211)	Proposed / Proposed
HCM-BEC-healthcare-location CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION	BEC CT3: Healthcare Location Profile Administration (p. 277)	Proposed / Proposed
HCM-BEC-healthcare-organisation CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION	BEC CT3: Healthcare Organisation Profile Administration (p. 265)	Proposed / Proposed
HCM-BEC-healthcare-service-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION	BEC CT3: Healthcare Service Profile Administration (p. 289)	Proposed / Proposed
HCM-BEC-healthcare-subject CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT	BEC CT3: Healthcare Subject Profile Administration (p. 226)	Canonical / Proposed
HCM-BEC-human-collaboration-work-enablement CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT	BEC CT1: Human Collaboration & Work Enablement (p. 924)	Proposed / Proposed
HCM-BEC-independent-assurance-evaluation-enablement CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT	BEC CT2: Independent Assurance Evaluation Enablement (p. 958)	Proposed / Proposed
HCM-BEC-independent-service-assurance-enablement CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT	BEC CT1: Independent Assurance Enablement (p. 821)	Proposed / Proposed
HCM-BEC-information-authority-representation CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-INFORMATION-AUTHORITY-REPRESENTATION	BEC CT3: Information Authority Representation (p. 959)	Proposed / Proposed
HCM-BEC-information-design-governance CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE	BEC CT2: Information Design Governance (p. 792)	Canonical / Proposed
HCM-BEC-information-lineage-stewardship CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP	BEC CT3: Data Provenance Capture (p. 733)	Proposed / Proposed

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HCM-BEC-information-protection-accountability-enablement CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT	BEC CT2: Information Protection & Accountability Enablement (p. 955)	Proposed / Proposed
HCM-BEC-inpatient-care CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	Canonical / Proposed
HCM-BEC-location-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION	BEC CT2: Location Administration (p. 276)	Canonical / Proposed
HCM-BEC-location-structure CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-LOCATION-STRUCTURE	BEC CT3: Location Structure (p. 930)	Proposed / Proposed
HCM-BEC-longitudinal-community-care-integration CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION	BEC CT2: Longitudinal & Community Care Integration (p. 942)	Proposed / Proposed
HCM-BEC-medication-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION	BEC CT2: Medication Administration (p. 417)	Canonical / Proposed
HCM-BEC-medication-therapy CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	Canonical / Proposed
HCM-BEC-mobile-staff-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT	BEC CT2: Mobile Staff Management (p. 591)	Canonical / Proposed
HCM-BEC-on-call-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT	BEC CT2: On-Call Management (p. 601)	Canonical / Proposed
HCM-BEC-operational-activity-temporal-enablement CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT	BEC CT2: Operational Activity & Temporal Enablement (p. 957)	Proposed / Proposed
HCM-BEC-operational-workforce-coordination CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION	BEC CT2: Operational Workforce Coordination (p. 948)	Proposed / Proposed
HCM-BEC-order-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION	BEC CT2: Order Administration (p. 383)	Canonical / Proposed
HCM-BEC-organisation-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION	BEC CT2: Organisation Administration (p. 263)	Canonical / Proposed
HCM-BEC-organisation-contact CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATION-CONTACT	BEC CT3: Organisation Contact (p. 928)	Proposed / Proposed
HCM-BEC-organisational-structure CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATIONAL-STRUCTURE	BEC CT3: Organisational Structure (p. 927)	Proposed / Proposed

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HCM-BEC-outpatient-operations CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS	BEC CT2: Outpatient Operations (p. 561)	Canonical / Proposed
HCM-BEC-outreach-care CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE	BEC CT3: Outreach Care Continuity (p. 466)	Canonical / Proposed
HCM-BEC-patient-clinical-record CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD	BEC CT2: Patient Clinical Record (p. 656)	Canonical / Proposed
HCM-BEC-patient-transport CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT	BEC CT2: Patient Transport (p. 632)	Canonical / Proposed
HCM-BEC-person-identity CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY	BEC CT3: Person Identity Administration (p. 214)	Canonical / Proposed
HCM-BEC-population-health-notification-enablement CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT	BEC CT2: Population Health Information Reporting (p. 817)	Proposed / Proposed
HCM-BEC-practitioner CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER	BEC CT3: Practitioner Profile Administration (p. 245)	Proposed / Proposed
HCM-BEC-practitioner-role CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE	BEC CT3: Practitioner Role Administration (p. 249)	Proposed / Proposed
HCM-BEC-presentation-services CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES	BEC CT2: Presentation Services (p. 786)	Canonical / Proposed
HCM-BEC-preventive-care CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE	BEC CT3: Screening and Immunisation Enablement (p. 329)	Canonical / Proposed
HCM-BEC-primary-care CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE	BEC CT3: Primary Care Coordination (p. 669)	Canonical / Proposed
HCM-BEC-procedure-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION	BEC CT2: Procedure Administration (p. 435)	Canonical / Proposed
HCM-BEC-product-batch-serial-tracking CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION.CT3-BEC-PRODUCT-BATCH-SERIAL-TRACKING	BEC CT3: Health Materiel Traceability Administration (p. 304)	Proposed / Proposed
HCM-BEC-product-formulary-catalogue CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-PRODUCT-ADMINISTRATION.CT3-BEC-PRODUCT-FORMULARY-CATALOGUE	BEC CT3: Health Materiel Catalogue & Formulary Administration (p. 302)	Proposed / Proposed
HCM-BEC-provider-administration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION	BEC CT2: Provider Administration (p. 243)	Canonical / Proposed

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HCM-BEC-provider-engagement CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-ENGAGEMENT	BEC CT3: Provider Engagement (p. 926)	Proposed / Proposed
HCM-BEC-provider-relationship CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-RELATIONSHIP	BEC CT3: Provider Relationship (p. 925)	Proposed / Proposed
HCM-BEC-referral-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION	BEC CT2: Referral Administration (p. 342)	Canonical / Proposed
HCM-BEC-rehabilitation CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-REHABILITATION	BEC CT2: Rehabilitation (p. 506)	Canonical / Proposed
HCM-BEC-remote-virtual-care CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE	BEC CT3: Remote Patient Monitoring (p. 675)	Canonical / Proposed
HCM-BEC-reviewed-activity-routing-management CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-WORKFLOW-ACTIVITY-COORDINATION.CT3-BEC-ACTIVITY-ROUTING-MANAGEMENT	BEC CT3: Activity Routing Management (p. 777)	Proposed / Proposed
HCM-BEC-reviewed-acute-care CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-ACUTE-CARE	BEC CT2: Acute Care (p. 495)	Proposed / Proposed
HCM-BEC-reviewed-appointment-booking-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION	BEC CT3: Appointment & Booking Administration (p. 357)	Proposed / Proposed
HCM-BEC-reviewed-assurance-service-evaluation CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SERVICE-REVIEW.CT3-BEC-ASSURANCE-SERVICE-EVALUATION	BEC CT3: Assurance Service Evaluation (p. 834)	Proposed / Proposed
HCM-BEC-reviewed-assurance-service-review CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SERVICE-REVIEW	BEC CT2: Assurance Service Review (p. 832)	Proposed / Proposed
HCM-BEC-reviewed-bed-care-place-state-administration CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-STATE-ADMINISTRATION	BEC CT3: Bed & Care-Place State Administration (p. 571)	Proposed / Proposed
HCM-BEC-reviewed-bed-care-place-turnover-coordination CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION	BEC CT3: Bed & Care-Place Turnover Coordination (p. 573)	Proposed / Proposed
HCM-BEC-reviewed-care-plan-lifecycle-coordination CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION	BEC CT3: Care Plan Lifecycle & Coordination (p. 457)	Proposed / Proposed

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HCM-BEC-reviewed-client-intake-demographic-update CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE	BEC CT3: Client Intake & Demographic Update (p. 336)	Proposed / Proposed
HCM-BEC-reviewed-clinical-context-collaboration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION	BEC CT3: Clinical Context Collaboration (p. 758)	Proposed / Proposed
HCM-BEC-reviewed-clinical-documentation-lifecycle-administration CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION	BEC CT3: Clinical Documentation Lifecycle Administration (p. 445)	Proposed / Proposed
HCM-BEC-reviewed-clinical-escalation-path-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-SUPERVISION.CT3-BEC-CLINICAL-ESCALATION-PATH-MANAGEMENT	BEC CT3: Clinical Escalation Path Management (p. 631)	Proposed / Proposed
HCM-BEC-reviewed-clinical-knowledge-artefact-distribution-management CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-ARTEFACT-DISTRIBUTION-MANAGEMENT	BEC CT3: Clinical Knowledge Artefact Distribution Management (p. 747)	Proposed / Proposed
HCM-BEC-reviewed-clinical-knowledge-request-administration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION	BEC CT3: Clinical Knowledge Request Administration (p. 744)	Proposed / Proposed
HCM-BEC-reviewed-clinical-roster-capture-communication CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-ROSTERING.CT3-BEC-CLINICAL-ROSTER-CAPTURE-COMMUNICATION	BEC CT3: Clinical Roster Capture & Communication (p. 590)	Proposed / Proposed
HCM-BEC-reviewed-clinical-service-credential-definition-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CREDENTIAL-MANAGEMENT.CT3-BEC-CLINICAL-SERVICE-CREDENTIAL-DEFINITION-MANAGEMENT	BEC CT3: Clinical Service Credential Definition Management (p. 629)	Proposed / Proposed
HCM-BEC-reviewed-clinical-service-dependency-management CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT	BEC CT1: Clinical Service Dependency Management (p. 491)	Proposed / Proposed
HCM-BEC-reviewed-closed-loop-order-progression CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION	BEC CT3: Closed-Loop Order Progression (p. 385)	Proposed / Proposed
HCM-BEC-reviewed-community-care CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-COMMUNITY-CARE	BEC CT2: Community Care (p. 508)	Proposed / Proposed

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HCM-BEC-reviewed-contextual-information-presentation CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.CT3-BEC-CONTEXTUAL-INFORMATION-PRESENTATION	BEC CT3: Contextual Information Presentation (p. 788)	Proposed / Proposed
HCM-BEC-reviewed-credential-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CREDENTIAL-MANAGEMENT	BEC CT2: Credential Management (p. 627)	Proposed / Proposed
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HCM-EC-governed-activity-execution-control CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10.CT3-EC-GOVERNED-ACTIVITY-EXECUTION-CONTROL	Enterprise CT3: Governed Activity Execution Control (p. 860)	Proposed / Proposed
HCM-EC-governed-information-operations CT1-EC-GOVERNED-INFORMATION-OPERATIONS	Enterprise CT1: Governed Information Operations (p. 837)	Proposed / Proposed
HCM-EC-information-attribution-lineage CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-07.CT3-EC-INFORMATION-ATTRIBUTION-LINEAGE	Enterprise CT3: Information Attribution & Lineage (p. 871)	Proposed / Proposed
HCM-EC-information-authority-qualification CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-INFORMATION-AUTHORITY-QUALIFICATION	Enterprise CT2: Information Authority Qualification (p. 877)	Proposed / Proposed
HCM-EC-operation-bound-policy-enforcement CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06.CT3-EC-OPERATION-BOUND-POLICY-ENFORCEMENT	Enterprise CT3: Operation-Bound Policy Enforcement (p. 868)	Proposed / Proposed
HCM-EC-operation-policy-resolution CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06.CT3-EC-OPERATION-POLICY-RESOLUTION	Enterprise CT3: Operation Policy Resolution (p. 867)	Proposed / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-EC-operational-fulfilment-definition CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10.CT3-EC-OPERATIONAL-FULFILMENT-DEFINITION	Enterprise CT3: Operational Fulfilment Definition (p. 859)	Proposed / Proposed
HCM-EC-trust-accountability-assurance CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE	Enterprise CT1: Trust, Accountability & Assurance (p. 864)	Proposed / Proposed
HCM-FN-accept-governed-dataset-revision CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FN-ACCEPT-GOVERNED-DATASET-REVISION	Function: Accept Governed Dataset Revision (p. 751)	Proposed / Proposed
HCM-FN-adjudicate-assurance-assessment CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FN-ADJUDICATE-ASSURANCE-ASSESSMENT	Function: Adjudicate Assurance Assessment (p. 912)	Canonical / Proposed
HCM-FN-aggregate-service-capacity-telemetry CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.FN-AGGREGATE-SERVICE-CAPACITY-TELEMETRY	Function: Aggregate Service Capacity Telemetry (p. 585)	Canonical / Proposed
HCM-FN-apply-access-qualified-result-filtering CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-APPLY-ACCESS-QUALIFIED-RESULT-FILTERING	Function: Apply Access-Qualified Result Filtering (p. 709)	Canonical / Proposed
HCM-FN-apply-governed-person-identity-correction CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-APPLY-GOVERNED-PERSON-IDENTITY-CORRECTION	Function: Apply Governed Person Identity Correction (p. 222)	Canonical / Proposed
HCM-FN-assemble-acute-clinical-baseline CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FN-ASSEMBLE-ACUTE-CLINICAL-BASELINE	Function: Assemble Acute Clinical Baseline (p. 368)	Canonical / Proposed
HCM-FN-assemble-associated-binary-content CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-ASSEMBLE-ASSOCIATED-BINARY-CONTENT	Function: Assemble Associated Binary Content (p. 667)	Proposed / Proposed
HCM-FN-assemble-inpatient-discharge-dossier CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FN-ASSEMBLE-INPATIENT-DISCHARGE-DOSSIER	Function: Assemble Inpatient Discharge Dossier (p. 534)	Canonical / Proposed
HCM-FN-assemble-longitudinal-clinical-record CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-ASSEMBLE-LONGITUDINAL-CLINICAL-RECORD	Function: Assemble Longitudinal Clinical Record (p. 681)	Canonical / Proposed

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HCM-FN-assemble-purpose-qualified-research-release CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FN-ASSEMBLE-PURPOSE-QUALIFIED-RESEARCH-RELEASE	Function: Assemble Purpose-Qualified Research Release (p. 814)	Proposed / Proposed
HCM-FN-assemble-referral-context CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-ASSEMBLE-REFERRAL-CONTEXT	Function: Assemble Referral Context (p. 351)	Canonical / Proposed
HCM-FN-assemble-ward-handover-context CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.FN-ASSEMBLE-WARD-HANDOVER-CONTEXT	Function: Assemble Ward Handover Context (p. 537)	Canonical / Proposed
HCM-FN-assess-assurance-evidence CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FN-ASSESS-ASSURANCE-EVIDENCE	Function: Assess Assurance Evidence (p. 911)	Canonical / Proposed
HCM-FN-assess-service-offering-readiness CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION.FN-ASSESS-SERVICE-OFFERING-READINESS	Function: Assess Service Offering Readiness (p. 994)	Proposed / Proposed
HCM-FN-associate-care-plan-context CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-ASSOCIATE-CARE-PLAN-CONTEXT	Function: Associate Care Plan Context (p. 473)	Canonical / Proposed
HCM-FN-associate-order-outcome CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-ASSOCIATE-ORDER-OUTCOME	Function: Associate Order Outcome (p. 396)	Canonical / Proposed
HCM-FN-bind-community-support-network-context CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FN-BIND-COMMUNITY-SUPPORT-NETWORK-CONTEXT	Function: Bind Community Support Network Context (p. 465)	Canonical / Proposed
HCM-FN-bind-diagnostic-report CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-BIND-DIAGNOSTIC-REPORT	Function: Bind Diagnostic Report (p. 414)	Canonical / Proposed
HCM-FN-bind-ed-triage-assessment CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FN-BIND-ED-TRIAGE-ASSESSMENT	Function: Bind ED Triage Assessment (p. 557)	Canonical / Proposed
HCM-FN-bind-practitioner-electronic-endpoints CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-BIND-PRACTITIONER-ELECTRONIC-ENDPOINTS	Function: Bind Practitioner Electronic Endpoints (p. 258)	Canonical / Proposed
HCM-FN-capture-post-clinic-follow-up-requirements CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.FN-CAPTURE-POST-CLINIC-FOLLOW-UP-REQUIREMENTS	Function: Capture Post-Clinic Follow-up Requirements (p. 569)	Canonical / Proposed

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HCM-FN-capture-required-significant-audit-evidence CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE.FN-CAPTURE-REQUIRED-SIGNIFICANT-AUDIT-EVIDENCE	Function: Capture Required Significant Audit Evidence (p. 984)	Proposed / Proposed
HCM-FN-consolidate-inpatient-clinical-round-feed CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FN-CONSOLIDATE-INPATIENT-CLINICAL-ROUND-FEED	Function: Consolidate Inpatient Clinical Round Feed (p. 533)	Canonical / Proposed
HCM-FN-coordinate-care-team-collaboration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-COORDINATE-CARE-TEAM-COLLABORATION	Function: Coordinate Care-Team Collaboration (p. 763)	Canonical / Proposed
HCM-FN-coordinate-ed-departmental-capacity CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FN-COORDINATE-ED-DEPARTMENTAL-CAPACITY	Function: Coordinate ED Departmental Capacity (p. 560)	Canonical / Proposed
HCM-FN-coordinate-entity-relative-input CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.FN-COORDINATE-ENTITY-RELATIVE-INPUT	Function: Coordinate Entity-Relative Input (p. 986)	Proposed / Proposed
HCM-FN-coordinate-multi-agency-chronic-care-plan CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FN-COORDINATE-MULTI-AGENCY-CHRONIC-CARE-PLAN	Function: Coordinate Multi-Agency Chronic Care Plan (p. 674)	Canonical / Proposed
HCM-FN-coordinate-operational-collaboration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.FN-COORDINATE-OPERATIONAL-COLLABORATION	Function: Coordinate Operational Collaboration (p. 772)	Canonical / Proposed
HCM-FN-coordinate-order-modification-cancellation CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-COORDINATE-ORDER-MODIFICATION-CANCELLATION	Function: Coordinate Order Modification / Cancellation (p. 395)	Canonical / Proposed
HCM-FN-coordinate-synthetic-task CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-COORDINATE-SYNTHETIC-TASK	Function: Coordinate Synthetic Task (p. 907)	Canonical / Proposed
HCM-FN-coordinate-to-do CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-COORDINATE-TO-DO	Function: Coordinate To Do (p. 905)	Canonical / Proposed
HCM-FN-coordinate-work-order CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-COORDINATE-WORK-ORDER	Function: Coordinate Work Order (p. 904)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-correlate-activity-with-temporal-context CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.FN-CORRELATE-ACTIVITY-WITH-TEMPORAL-CONTEXT	Function: Correlate Activity with Temporal Context (p. 785)	Canonical / Proposed
HCM-FN-correlate-adverse-reaction-allergy-profile CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FN-CORRELATE-ADVERSE-REACTION-ALLERGY-PROFILE	Function: Correlate Adverse Reaction & Allergy Profile (p. 424)	Canonical / Proposed
HCM-FN-correlate-diagnostic-request CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-CORRELATE-DIAGNOSTIC-REQUEST	Function: Correlate Diagnostic Request (p. 413)	Canonical / Proposed
HCM-FN-correlate-person-identifiers CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-CORRELATE-PERSON-IDENTIFIERS	Function: Correlate Person Identifiers (p. 220)	Canonical / Proposed
HCM-FN-correlate-primary-care-consultation-context CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FN-CORRELATE-PRIMARY-CARE-CONSULTATION-CONTEXT	Function: Correlate Primary Care Consultation Context (p. 672)	Canonical / Proposed
HCM-FN-correlate-virtual-care-consultation-context CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-CORRELATE-VIRTUAL-CARE-CONSULTATION-CONTEXT	Function: Correlate Virtual Care Consultation Context (p. 679)	Canonical / Proposed
HCM-FN-design-service-assurance CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN.FN-DESIGN-SERVICE-ASSURANCE	Function: Design Service Assurance (p. 826)	Canonical / Proposed
HCM-FN-determine-reconciled-activity-continuation CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-DETERMINE-RECONCILED-ACTIVITY-CONTINUATION	Function: Determine Reconciled Activity Continuation (p. 992)	Proposed / Proposed
HCM-FN-dispatch-operational-work-order CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.FN-DISPATCH-OPERATIONAL-WORK-ORDER	Function: Dispatch Operational Work Order (p. 614)	Canonical / Proposed
HCM-FN-distribute-addressed-information CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-DISTRIBUTE-ADDRESSED-INFORMATION	Function: Distribute Addressed Information (p. 698)	Canonical / Proposed
HCM-FN-distribute-care-plan-change CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-DISTRIBUTE-CARE-PLAN-CHANGE	Function: Distribute Care Plan Change (p. 474)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-distribute-clinical-communication CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-DISTRIBUTE-CLINICAL-COMMUNICATION	Function: Distribute Clinical Communication (p. 482)	Canonical / Proposed
HCM-FN-distribute-diagnostic-report CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-DISTRIBUTE-DIAGNOSTIC-REPORT	Function: Distribute Diagnostic Report (p. 416)	Canonical / Proposed
HCM-FN-distribute-governed-demographic-change CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-DISTRIBUTE-GOVERNED-DEMOGRAPHIC-CHANGE	Function: Distribute Governed Demographic Change (p. 341)	Canonical / Proposed
HCM-FN-establish-activity-effect-observation-basis CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-ESTABLISH-ACTIVITY-EFFECT-OBSERVATION-BASIS	Function: Establish Activity Effect Observation Basis (p. 991)	Proposed / Proposed
HCM-FN-establish-assurance-context CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FN-ESTABLISH-ASSURANCE-CONTEXT	Function: Establish Assurance Context (p. 909)	Canonical / Proposed
HCM-FN-establish-encounter-context CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-ESTABLISH-ENCOUNTER-CONTEXT	Function: Establish Encounter Context (p. 377)	Canonical / Proposed
HCM-FN-establish-healthcare-subject-context CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FN-ESTABLISH-HEALTHCARE-SUBJECT-CONTEXT	Function: Establish Healthcare Subject Context (p. 229)	Canonical / Proposed
HCM-FN-evaluate-client-confidentiality-restrictions CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FN-EVALUATE-CLIENT-CONFIDENTIALITY-RESTRICTIONS	Function: Evaluate Client Confidentiality Restrictions (p. 242)	Canonical / Proposed
HCM-FN-evaluate-client-consent CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.FN-EVALUATE-CLIENT-CONSENT	Function: Evaluate Client Consent (p. 240)	Canonical / Proposed
HCM-FN-evaluate-consent-constraint CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-CONSENT-CONSTRAINT	Function: Evaluate Consent Constraint (p. 724)	Canonical / Proposed
HCM-FN-evaluate-information-access-authority CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-INFORMATION-ACCESS-AUTHORITY	Function: Evaluate Information Access Authority (p. 722)	Canonical / Proposed

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HCM-FN-govern-client-relationship-validity CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FN-GOVERN-CLIENT-RELATIONSHIP-VALIDITY	Function: Govern Client Relationship Validity (p. 236)	Canonical / Proposed
HCM-FN-govern-clinical-document-lifecycle CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-GOVERN-CLINICAL-DOCUMENT-LIFECYCLE	Function: Govern Clinical Document Lifecycle (p. 451)	Canonical / Proposed
HCM-FN-govern-collaboration-metadata CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-GOVERN-COLLABORATION-METADATA	Function: Govern Collaboration Metadata (p. 766)	Canonical / Proposed
HCM-FN-govern-exchange-profile CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-EXCHANGE-PROFILE	Function: Govern Exchange Profile (p. 807)	Canonical / Proposed
HCM-FN-govern-information-definition CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-DEFINITION	Function: Govern Information Definition (p. 804)	Canonical / Proposed
HCM-FN-govern-information-semantics CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-SEMANTICS	Function: Govern Information Semantics (p. 803)	Canonical / Proposed
HCM-FN-govern-organisation-profile CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-GOVERN-ORGANISATION-PROFILE	Function: Govern Organisation Profile (p. 272)	Canonical / Proposed
HCM-FN-govern-practitioner-profile CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-GOVERN-PRACTITIONER-PROFILE	Function: Govern Practitioner Profile (p. 256)	Canonical / Proposed
HCM-FN-govern-subject-demographic-context CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.FN-GOVERN-SUBJECT-DEMOGRAPHIC-CONTEXT	Function: Govern Subject Demographic Context (p. 230)	Canonical / Proposed
HCM-FN-govern-terminology-binding CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-TERMINOLOGY-BINDING	Function: Govern Terminology Binding (p. 806)	Canonical / Proposed
HCM-FN-index-clinical-document-metadata CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-INDEX-CLINICAL-DOCUMENT-METADATA	Function: Index Clinical Document Metadata (p. 452)	Canonical / Proposed
HCM-FN-ingest-remote-patient-telemetry CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-INGEST-REMOTE-PATIENT-TELEMETRY	Function: Ingest Remote Patient Telemetry (p. 680)	Canonical / Proposed
HCM-FN-integrate-mobile-work-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-INTEGRATE-MOBILE-WORK-MANAGEMENT	Function: Integrate Mobile Work Management (p. 601)	Canonical / Proposed

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HCM-FN-maintain-active-clinical-record CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-MAINTAIN-ACTIVE-CLINICAL-RECORD	Function: Maintain Active Clinical Record (p. 683)	Canonical / Proposed
HCM-FN-maintain-care-place-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.FN-MAINTAIN-CARE-PLACE-DEFINITION	Function: Maintain Care-Place Definition (p. 286)	Canonical / Proposed
HCM-FN-maintain-client-relationships CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.FN-MAINTAIN-CLIENT-RELATIONSHIPS	Function: Maintain Client Relationships (p. 235)	Canonical / Proposed
HCM-FN-maintain-clinical-privileges CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-MAINTAIN-CLINICAL-PRIVILEGES	Function: Maintain Clinical Privileges (p. 261)	Canonical / Proposed
HCM-FN-maintain-device-association CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-ASSOCIATION	Function: Maintain Device Association (p. 322)	Canonical / Proposed
HCM-FN-maintain-device-communication-endpoints CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-COMMUNICATION-ENDPOINTS	Function: Maintain Device Communication Endpoints (p. 323)	Canonical / Proposed
HCM-FN-maintain-device-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-DEFINITION	Function: Maintain Device Definition (p. 320)	Canonical / Proposed
HCM-FN-maintain-device-instance-identity CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.FN-MAINTAIN-DEVICE-INSTANCE-IDENTITY	Function: Maintain Device Instance Identity (p. 321)	Canonical / Proposed
HCM-FN-maintain-encounter-care-place-association CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-MAINTAIN-ENCOUNTER-CARE-PLACE-ASSOCIATION	Function: Maintain Encounter Care-Place Association (p. 380)	Canonical / Proposed
HCM-FN-maintain-healthcare-location CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.FN-MAINTAIN-HEALTHCARE-LOCATION	Function: Maintain Healthcare Location (p. 283)	Canonical / Proposed
HCM-FN-maintain-healthcare-service-definition CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-HEALTHCARE-SERVICE-DEFINITION	Function: Maintain Healthcare Service Definition (p. 294)	Canonical / Proposed
HCM-FN-maintain-operational-bed-state CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MAINTAIN-OPERATIONAL-BED-STATE	Function: Maintain Operational Bed State (p. 576)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-maintain-operational-fulfilment-definition CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN.FN-MAINTAIN-OPERATIONAL-FULFILMENT-DEFINITION	Function: Maintain Operational Fulfilment Definition (p. 988)	Proposed / Proposed
HCM-FN-maintain-organisation-contacts CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-MAINTAIN-ORGANISATION-CONTACTS	Function: Maintain Organisation Contacts (p. 274)	Canonical / Proposed
HCM-FN-maintain-organisational-structure CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-MAINTAIN-ORGANISATIONAL-STRUCTURE	Function: Maintain Organisational Structure (p. 273)	Canonical / Proposed
HCM-FN-maintain-person-identity-aliases CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-MAINTAIN-PERSON-IDENTITY-ALIASES	Function: Maintain Person Identity Aliases (p. 222)	Canonical / Proposed
HCM-FN-maintain-physical-location-hierarchy CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.FN-MAINTAIN-PHYSICAL-LOCATION-HIERARCHY	Function: Maintain Physical Location Hierarchy (p. 284)	Canonical / Proposed
HCM-FN-maintain-practitioner-affiliations CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-MAINTAIN-PRACTITIONER-AFFILIATIONS	Function: Maintain Practitioner Affiliations (p. 260)	Canonical / Proposed
HCM-FN-maintain-practitioner-roles CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-MAINTAIN-PRACTITIONER-ROLES	Function: Maintain Practitioner Roles (p. 257)	Canonical / Proposed
HCM-FN-maintain-service-availability CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-SERVICE-AVAILABILITY	Function: Maintain Service Availability (p. 297)	Canonical / Proposed
HCM-FN-maintain-service-eligibility-rules CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-SERVICE-ELIGIBILITY-RULES	Function: Maintain Service Eligibility Rules (p. 298)	Canonical / Proposed
HCM-FN-maintain-service-location-provider-map CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.FN-MAINTAIN-SERVICE-LOCATION-PROVIDER-MAP	Function: Maintain Service / Location / Provider Map (p. 296)	Canonical / Proposed
HCM-FN-manage-assurance-criteria CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-CRITERIA-MANAGEMENT.FN-MANAGE-ASSURANCE-CRITERIA	Function: Manage Assurance Criteria (p. 830)	Canonical / Proposed
HCM-FN-manage-bed-turnover-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MANAGE-BED-TURNOVER-PROGRESSION	Function: Manage Bed Turnover Progression (p. 578)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-manage-clinic-practice-capacity CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-PRACTICE-CAPACITY	Function: Manage Clinic & Practice Capacity (p. 520)	Canonical / Proposed
HCM-FN-manage-clinic-queue-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-QUEUE-PROGRESSION	Function: Manage Clinic Queue Progression (p. 521)	Canonical / Proposed
HCM-FN-manage-discharge-coordination-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.FN-MANAGE-DISCHARGE-COORDINATION-PROGRESSION	Function: Manage Discharge Coordination Progression (p. 651)	Canonical / Proposed
HCM-FN-manage-mobile-service-capacity CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-MANAGE-MOBILE-SERVICE-CAPACITY	Function: Manage Mobile Service Capacity (p. 597)	Canonical / Proposed
HCM-FN-manage-operational-work-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.FN-MANAGE-OPERATIONAL-WORK-PROGRESSION	Function: Manage Operational Work Progression (p. 615)	Canonical / Proposed
HCM-FN-manage-order-progression CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-MANAGE-ORDER-PROGRESSION	Function: Manage Order Progression (p. 393)	Canonical / Proposed
HCM-FN-manage-outpatient-capacity CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.FN-MANAGE-OUTPATIENT-CAPACITY	Function: Manage Outpatient Capacity (p. 567)	Canonical / Proposed
HCM-FN-manage-patient-transport-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.FN-MANAGE-PATIENT-TRANSPORT-PROGRESSION	Function: Manage Patient Transport Progression (p. 637)	Canonical / Proposed
HCM-FN-manage-referral-progression CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-MANAGE-REFERRAL-PROGRESSION	Function: Manage Referral Progression (p. 352)	Canonical / Proposed
HCM-FN-manage-specimen-transport-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.FN-MANAGE-SPECIMEN-TRANSPORT-PROGRESSION	Function: Manage Specimen Transport Progression (p. 902)	Canonical / Proposed
HCM-FN-manage-theatre-capacity CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CAPACITY	Function: Manage Theatre Capacity (p. 544)	Canonical / Proposed
HCM-FN-manage-theatre-case-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CASE-PROGRESSION	Function: Manage Theatre Case Progression (p. 545)	Canonical / Proposed
HCM-FN-manage-ward-capacity CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.FN-MANAGE-WARD-CAPACITY	Function: Manage Ward Capacity (p. 536)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-map-clinical-concept CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-MAP-CLINICAL-CONCEPT	Function: Map Clinical Concept (p. 755)	Canonical / Proposed
HCM-FN-mediate-information-retrieval CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-MEDIATE-INFORMATION-RETRIEVAL	Function: Mediate Information Retrieval (p. 697)	Canonical / Proposed
HCM-FN-mediate-standards-based-health-information-communication CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION.FN-MEDIATE-STANDARDS-BASED-HEALTH-INFORMATION-COMMUNICATION	Function: Mediate Standards-Based Health Information Communication (p. 902)	Canonical / Proposed
HCM-FN-present-contextual-health-and-operational-information CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.FN-PRESENT-CONTEXTUAL-HEALTH-AND-OPERATIONAL-INFORMATION	Function: Present Contextual Health and Operational Information (p. 790)	Canonical / Proposed
HCM-FN-preserve-durable-clinical-record CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-PRESERVE-DURABLE-CLINICAL-RECORD	Function: Preserve Durable Clinical Record (p. 684)	Canonical / Proposed
HCM-FN-progress-encounter-state CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-PROGRESS-ENCOUNTER-STATE	Function: Progress Encounter State (p. 378)	Canonical / Proposed
HCM-FN-project-longitudinal-medication-history CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FN-PROJECT-LONGITUDINAL-MEDICATION-HISTORY	Function: Project Longitudinal Medication History (p. 423)	Canonical / Proposed
HCM-FN-project-operational-schedule CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.FN-PROJECT-OPERATIONAL-SCHEDULE	Function: Project Operational Schedule (p. 783)	Canonical / Proposed
HCM-FN-project-screening-eligibility-timeline CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FN-PROJECT-SCREENING-ELIGIBILITY-TIMELINE	Function: Project Screening Eligibility Timeline (p. 334)	Canonical / Proposed
HCM-FN-propagate-security-context CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-PROPAGATE-SECURITY-CONTEXT	Function: Propagate Security Context (p. 725)	Canonical / Proposed
HCM-FN-provide-emergency-lhr-summary CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FN-PROVIDE-EMERGENCY-LHR-SUMMARY	Function: Provide Emergency LHR Summary (p. 558)	Canonical / Proposed
HCM-FN-provide-outreach-visit-context CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FN-PROVIDE-OUTREACH-VISIT-CONTEXT	Function: Provide Outreach Visit Context (p. 470)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-publish-governed-diagnostic-report CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-PUBLISH-GOVERNED-DIAGNOSTIC-REPORT	Function: Publish Governed Diagnostic Report (p. 406)	Canonical / Proposed
HCM-FN-qualify-information-authority-standing CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-INFORMATION-AUTHORITY-REPRESENTATION.FN-QUALIFY-INFORMATION-AUTHORITY-STANDING	Function: Qualify Information Authority Standing (p. 983)	Proposed / Proposed
HCM-FN-receive-appointment-notification CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.FN-RECEIVE-APPOINTMENT-NOTIFICATION	Function: Receive Appointment Notification (p. 360)	Canonical / Proposed
HCM-FN-receive-care-plan CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-RECEIVE-CARE-PLAN	Function: Receive Care Plan (p. 472)	Canonical / Proposed
HCM-FN-receive-client-demographic-change CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-RECEIVE-CLIENT-DEMOGRAPHIC-CHANGE	Function: Receive Client Demographic Change (p. 339)	Canonical / Proposed
HCM-FN-receive-clinical-document CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-RECEIVE-CLINICAL-DOCUMENT	Function: Receive Clinical Document (p. 450)	Canonical / Proposed
HCM-FN-receive-diagnostic-requisition-feed CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-RECEIVE-DIAGNOSTIC-REQUISITION-FEED	Function: Receive Diagnostic Requisition Feed (p. 404)	Canonical / Proposed
HCM-FN-receive-information-submission CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-RECEIVE-INFORMATION-SUBMISSION	Function: Receive Information Submission (p. 695)	Canonical / Proposed
HCM-FN-receive-medication-administration-event CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ADMINISTRATION-EVENT	Function: Receive Medication Administration Event (p. 434)	Canonical / Proposed
HCM-FN-receive-medication-order CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ORDER	Function: Receive Medication Order (p. 431)	Canonical / Proposed
HCM-FN-receive-mobile-visit-progression CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-RECEIVE-MOBILE-VISIT-PROGRESSION	Function: Receive Mobile Visit Progression (p. 600)	Canonical / Proposed
HCM-FN-receive-order-request CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-RECEIVE-ORDER-REQUEST	Function: Receive Order Request (p. 391)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-receive-procedural-documentation CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURAL-DOCUMENTATION	Function: Receive Procedural Documentation (p. 442)	Canonical / Proposed
HCM-FN-receive-procedure-booking-request CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURE-BOOKING-REQUEST	Function: Receive Procedure Booking / Request (p. 440)	Canonical / Proposed
HCM-FN-receive-referral CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-RECEIVE-REFERRAL	Function: Receive Referral (p. 349)	Canonical / Proposed
HCM-FN-reconcile-indeterminate-activity-effects CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-RECONCILE-INDETERMINATE-ACTIVITY-EFFECTS	Function: Reconcile Indeterminate Activity Effects (p. 990)	Proposed / Proposed
HCM-FN-reconcile-outreach-clinical-batch CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FN-RECONCILE-OUTREACH-CLINICAL-BATCH	Function: Reconcile Outreach Clinical Batch (p. 470)	Canonical / Proposed
HCM-FN-record-communication-transaction CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-RECORD-COMMUNICATION-TRANSACTION	Function: Record Communication Transaction (p. 485)	Canonical / Proposed
HCM-FN-record-security-significant-activity CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-RECORD-SECURITY-SIGNIFICANT-ACTIVITY	Function: Record Security-Significant Activity (p. 727)	Canonical / Proposed
HCM-FN-record-significant-information-lineage CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FN-RECORD-SIGNIFICANT-INFORMATION-LINEAGE	Function: Record Significant Information Lineage (p. 736)	Proposed / Proposed
HCM-FN-resolve-active-on-call-coverage CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.FN-RESOLVE-ACTIVE-ON-CALL-COVERAGE	Function: Resolve Active On-Call Coverage (p. 605)	Canonical / Proposed
HCM-FN-resolve-associated-binary-content CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-RESOLVE-ASSOCIATED-BINARY-CONTENT	Function: Resolve Associated Binary Content (p. 666)	Proposed / Proposed
HCM-FN-resolve-clinical-concept CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-RESOLVE-CLINICAL-CONCEPT	Function: Resolve Clinical Concept (p. 753)	Canonical / Proposed
HCM-FN-resolve-lhr-query-within-collaboration CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-RESOLVE-LHR-QUERY-WITHIN-COLLABORATION	Function: Resolve LHR Query within Collaboration (p. 765)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-resolve-order-destination CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-RESOLVE-ORDER-DESTINATION	Function: Resolve Order Destination (p. 392)	Canonical / Proposed
HCM-FN-resolve-person-identifier CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FN-RESOLVE-PERSON-IDENTIFIER	Function: Resolve Person Identifier (p. 219)	Canonical / Proposed
HCM-FN-retrieve-health-information CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-RETRIEVE-HEALTH-INFORMATION	Function: Retrieve Health Information (p. 708)	Canonical / Proposed
HCM-FN-route-acute-critical-result-alert CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FN-ROUTE-ACUTE-CRITICAL-RESULT-ALERT	Function: Route Acute Critical Result Alert (p. 370)	Canonical / Proposed
HCM-FN-search-health-information CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-SEARCH-HEALTH-INFORMATION	Function: Search Health Information (p. 706)	Canonical / Proposed
HCM-FN-supervise-activity-timeout-escalation CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FN-SUPERVISE-ACTIVITY-TIMEOUT-ESCALATION	Function: Supervise Activity Timeout / Escalation (p. 908)	Canonical / Proposed
HCM-FN-synchronise-appointment-status CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.FN-SYNCHRONISE-APPOINTMENT-STATUS	Function: Synchronise Appointment Status (p. 361)	Canonical / Proposed
HCM-FN-synchronize-community-visit-record CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FN-SYNCHRONIZE-COMMUNITY-VISIT-RECORD	Function: Synchronize Community Visit Record (p. 463)	Canonical / Proposed
HCM-FN-syndicate-immunisation-record CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FN-SYNDICATE-IMMUNISATION-RECORD	Function: Syndicate Immunisation Record (p. 333)	Canonical / Proposed
HCM-FN-syndicate-information-change CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-SYNDICATE-INFORMATION-CHANGE	Function: Syndicate Information Change (p. 700)	Canonical / Proposed
HCM-FN-track-business-delivery-acknowledgement CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-TRACK-BUSINESS-DELIVERY-ACKNOWLEDGEMENT	Function: Track Business Delivery Acknowledgement (p. 484)	Canonical / Proposed
HCM-FN-track-clinic-operating-session CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-TRACK-CLINIC-OPERATING-SESSION	Function: Track Clinic Operating Session (p. 520)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FN-track-dispense-event CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-TRACK-DISPENSE-EVENT	Function: Track Dispense Event (p. 432)	Canonical / Proposed
HCM-FN-track-mobile-clinical-asset-operational-state CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FN-TRACK-MOBILE-CLINICAL-ASSET-OPERATIONAL-STATE	Function: Track Mobile Clinical Asset Operational State (p. 314)	Canonical / Proposed
HCM-FN-track-operational-staff-presence CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-TRACK-OPERATIONAL-STAFF-PRESENCE	Function: Track Operational Staff Presence (p. 598)	Canonical / Proposed
HCM-FN-verify-clinical-qualification-for-work-dispatch CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FN-VERIFY-CLINICAL-QUALIFICATION-FOR-WORK-DISPATCH	Function: Verify Clinical Qualification for Work Dispatch (p. 627)	Canonical / Proposed
HCM-FN-verify-healthcare-organisation CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.FN-VERIFY-HEALTHCARE-ORGANISATION	Function: Verify Healthcare Organisation (p. 270)	Canonical / Proposed
HCM-FN-verify-operational-access-credential CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CREDENTIAL-MANAGEMENT.FN-VERIFY-OPERATIONAL-ACCESS-CREDENTIAL	Function: Verify Operational Access Credential (p. 900)	Canonical / Proposed
HCM-FN-verify-practitioner-registration CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.FN-VERIFY-PRACTITIONER-REGISTRATION	Function: Verify Practitioner Registration (p. 254)	Canonical / Proposed
HCM-FN-verify-semantic-conformance CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-VERIFY-SEMANTIC-CONFORMANCE	Function: Verify Semantic Conformance (p. 808)	Canonical / Proposed
HCM-FT-activity-outcome-continuation-rules CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN.FEAT-STEP2-ACTIVITY-OUTCOME-CONTINUATION-RULES	Feature: Activity Outcome & Continuation Rules (p. 976)	Proposed / Proposed
HCM-FT-assurance-disposition-design-modelling CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN.FEAT-STEP2-ASSURANCE-DISPOSITION-DESIGN-MODELLING	Feature: Assurance Disposition & Design Modelling (p. 828)	Proposed / Proposed
HCM-FT-authorised-research-release-assembly CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-AUTHORISED-RESEARCH-RELEASE-ASSEMBLY	Feature: Authorised Research Release Assembly (p. 739)	Proposed / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-FT-binary-content-resolution-assembly CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FEAT-STEP2-BINARY-CONTENT-RESOLUTION-ASSEMBLY	Feature: Binary Content Resolution & Assembly (p. 665)	Proposed / Proposed
HCM-FT-care-goal-plan-progression-representation CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION.FEAT-STEP2-CARE-GOAL-PLAN-PROGRESSION-REPRESENTATION	Feature: Care Goal & Plan Progression Representation (p. 459)	Proposed / Proposed
HCM-FT-clinic-session-capacity-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-STEP2-CLINIC-SESSION-CAPACITY-MANAGEMENT	Feature: Clinic Session Capacity Management (p. 519)	Proposed / Proposed
HCM-FT-clinical-information-historical-reconstruction CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-STEP2-CLINICAL-INFORMATION-HISTORICAL-RECONSTRUCTION	Feature: Clinical Information Historical Reconstruction (p. 662)	Proposed / Proposed
HCM-FT-collaboration-interaction-capture-qualification CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-STEP2-COLLABORATION-INTERACTION-CAPTURE-QUALIFICATION	Feature: Collaboration Interaction Capture Qualification (p. 762)	Proposed / Proposed
HCM-FT-controlled-administrative-recovery CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-STEP2-CONTROLLED-ADMINISTRATIVE-RECOVERY	Feature: Controlled Administrative Recovery (p. 978)	Proposed / Proposed
HCM-FT-demand-capacity-planning-projection CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-DEMAND-CAPACITY-PLANNING-PROJECTION	Feature: Demand & Capacity Planning Projection (p. 741)	Proposed / Proposed
HCM-FT-device-association-lineage CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-STEP2-DEVICE-ASSOCIATION-LINEAGE	Feature: Device Association Lineage (p. 315)	Proposed / Proposed
HCM-FT-diagnostic-preliminary-corrected-report-handling CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-STEP2-DIAGNOSTIC-PRELIMINARY-CORRECTED-REPORT-HANDLING	Feature: Diagnostic Preliminary & Corrected Report Handling (p. 412)	Proposed / Proposed

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HCM-FT-emergency-treatment-capacity-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-STEP2-EMERGENCY-TREATMENT-CAPACITY-MANAGEMENT	Feature: Emergency Treatment Capacity Management (p. 553)	Proposed / Proposed
HCM-FT-encounter-history-retrieval CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-STEP2-ENCOUNTER-HISTORY-RETRIEVAL	Feature: Encounter History Retrieval (p. 376)	Proposed / Proposed
HCM-FT-entity-behaviour-failure-recovery-governance CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.FEAT-STEP2-ENTITY-BEHAVIOUR-FAILURE-RECOVERY-GOVERNANCE	Feature: Entity Behaviour Failure & Recovery Governance (p. 974)	Proposed / Proposed
HCM-FT-entity-relative-activity-coordination CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.FEAT-STEP2-ENTITY-RELATIVE-ACTIVITY-COORDINATION	Feature: Entity-Relative Activity Coordination (p. 973)	Proposed / Proposed
HCM-FT-exchange-profile-governance CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-EXCHANGE-PROFILE-GOVERNANCE	Feature: Exchange Profile Governance (p. 798)	Proposed / Proposed
HCM-FT-governance-requirement-version-publication CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-GOVERNANCE-REQUIREMENT-PUBLICATION-SUPPORT.FEAT-STEP2-GOVERNANCE-REQUIREMENT-VERSION-PUBLICATION	Feature: Governance Requirement Version Publication (p. 981)	Proposed / Proposed
HCM-FT-governed-dataset-content-revision CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-STEP2-GOVERNED-DATASET-CONTENT-REVISION	Feature: Governed Dataset Content Revision (p. 750)	Proposed / Proposed
HCM-FT-governed-policy-assignment-revocation CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-STEP2-GOVERNED-POLICY-ASSIGNMENT-REVOCATION	Feature: Governed Policy Assignment & Revocation (p. 970)	Proposed / Proposed
HCM-FT-identity-source-correction-lineage CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.FEAT-STEP2-IDENTITY-SOURCE-CORRECTION-LINEAGE	Feature: Identity Source & Correction Lineage (p. 225)	Proposed / Proposed

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HCM-FT-independent-evidence-assessment-adjudication CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.FEAT-STEP2-INDEPENDENT-EVIDENCE-ASSESSMENT-ADJUDICATION	Feature: Independent Evidence Assessment & Adjudication (p. 982)	Proposed / Proposed
HCM-FT-information-authority-source-standing-representation CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-INFORMATION-AUTHORITY-REPRESENTATION.FEAT-STEP2-INFORMATION-AUTHORITY-SOURCE-STANDING-REPRESENTATION	Feature: Information Authority & Source Standing Representation (p. 967)	Proposed / Proposed
HCM-FT-information-definition-schema-governance CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-INFORMATION-DEFINITION-SCHEMA-GOVERNANCE	Feature: Information Definition & Schema Governance (p. 796)	Proposed / Proposed
HCM-FT-information-transformation-lineage CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FEAT-STEP2-INFORMATION-TRANSFORMATION-LINEAGE	Feature: Information Transformation Lineage (p. 735)	Proposed / Proposed
HCM-FT-mandatory-significant-activity-audit-capture CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE.FEAT-STEP2-MANDATORY-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE	Feature: Mandatory Significant Activity Audit Capture (p. 968)	Proposed / Proposed
HCM-FT-mobile-service-capacity-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-STEP2-MOBILE-SERVICE-CAPACITY-MANAGEMENT	Feature: Mobile Service Capacity Management (p. 596)	Proposed / Proposed
HCM-FT-operation-bound-authorisation-enforcement CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-STEP2-OPERATION-BOUND-AUTHORISATION-ENFORCEMENT	Feature: Operation-Bound Authorisation Enforcement (p. 972)	Proposed / Proposed
HCM-FT-operational-activity-context-definition CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-GOVERNED-OPERATIONAL-ACTIVITY-DESIGN.FEAT-STEP2-OPERATIONAL-ACTIVITY-CONTEXT-DEFINITION	Feature: Operational Activity Context Definition (p. 975)	Proposed / Proposed
HCM-FT-order-outcome-association CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-STEP2-ORDER-OUTCOME-ASSOCIATION	Feature: Order Outcome Association (p. 390)	Proposed / Proposed

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HCM-FT-outpatient-appointment-capacity-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-STEP2-OUTPATIENT-APPOINTMENT-CAPACITY-MANAGEMENT	Feature: Outpatient Appointment Capacity Management (p. 566)	Proposed / Proposed
HCM-FT-population-notification-obligation-tracking CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-POPULATION-NOTIFICATION-OBLIGATION-TRACKING	Feature: Population Notification Obligation Tracking (p. 740)	Proposed / Proposed
HCM-FT-provider-directory-discovery-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER.FEAT-STEP2-PROVIDER-DIRECTORY-DISCOVERY-RESOLUTION	Feature: Provider Directory Discovery & Resolution (p. 248)	Proposed / Proposed
HCM-FT-referral-destination-coordination CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-STEP2-REFERRAL-DESTINATION-COORDINATION	Feature: Referral Destination Coordination (p. 348)	Proposed / Proposed
HCM-FT-service-readiness-dependency-assessment CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION.FEAT-STEP2-SERVICE-READINESS-DEPENDENCY-ASSESSMENT	Feature: Service Readiness Dependency Assessment (p. 979)	Proposed / Proposed
HCM-FT-temporal-assurance-criteria-stewardship CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-CRITERIA-MANAGEMENT.FEAT-STEP2-TEMPORAL-ASSURANCE-CRITERIA-STEWARDSHIP	Feature: Temporal Assurance Criteria Stewardship (p. 831)	Proposed / Proposed
HCM-FT-terminology-binding-governance CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-TERMINOLOGY-BINDING-GOVERNANCE	Feature: Terminology Binding Governance (p. 797)	Proposed / Proposed
HCM-FT-theatre-session-capacity-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-STEP2-THEATRE-SESSION-CAPACITY-MANAGEMENT	Feature: Theatre Session Capacity Management (p. 543)	Proposed / Proposed
HCM-FT-trusted-identity-context-intake CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-STEP2-TRUSTED-IDENTITY-CONTEXT-INTAKE	Feature: Trusted Identity Context Intake (p. 969)	Proposed / Proposed

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HCM-FT-uncertain-activity-outcome-reconciliation CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.FEAT-STEP2-UNCERTAIN-ACTIVITY-OUTCOME-RECONCILIATION	Feature: Uncertain Activity Outcome Reconciliation (p. 977)	Proposed / Proposed
HCM-FT-ward-care-capacity-management CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-STEP2-WARD-CARE-CAPACITY-MANAGEMENT	Feature: Ward Care Capacity Management (p. 528)	Proposed / Proposed
HCM-PR-assurance-process-design CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-ASSURANCE-SPECIFICATION-ENABLEMENT.CT3-BEC-ASSURANCE-DESIGN.PR-ASSURANCE-PROCESS-DESIGN	Process: Assurance Process Design (p. 827)	Canonical / Proposed
HCM-PR-assurance-process-execution CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.PR-ASSURANCE-PROCESS-EXECUTION	Process: Assurance Process Execution (p. 920)	Canonical / Proposed
HCM-PR-assurance-process-reporting-communication CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.PR-ASSURANCE-PROCESS-REPORTING-COMMUNICATION	Process: Assurance Process Reporting / Communication (p. 922)	Canonical / Proposed
HCM-PR-bed-turnover-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.PR-BED-TURNOVER-PROCESS	Process: Bed Turnover Process (p. 580)	Canonical / Proposed
HCM-PR-clinic-operational-queue-progression-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.PR-CLINIC-OPERATIONAL-QUEUE-PROGRESSION-PROCESS	Process: Clinic / Operational Queue Progression Process (p. 523)	Canonical / Proposed
HCM-PR-clinical-document-lifecycle-process CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.PR-CLINICAL-DOCUMENT-LIFECYCLE-PROCESS	Process: Clinical Document Lifecycle Process (p. 454)	Canonical / Proposed
HCM-PR-closed-loop-order-progression-process CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.PR-CLOSED-LOOP-ORDER-PROGRESSION-PROCESS	Process: Closed-Loop Order Progression Process (p. 398)	Canonical / Proposed
HCM-PR-discharge-progression-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.PR-DISCHARGE-PROGRESSION-PROCESS	Process: Discharge Progression Process (p. 652)	Canonical / Proposed
HCM-PR-encounter-lifecycle-process CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.PR-ENCOUNTER-LIFECYCLE-PROCESS	Process: Encounter Lifecycle Process (p. 381)	Canonical / Proposed

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HCM-PR-governed-person-identity-correction-process CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.PR-GOVERNED-PERSON-IDENTITY-CORRECTION-PROCESS	Process: Governed Person Identity Correction Process (p. 224)	Canonical / Proposed
HCM-PR-indeterminate-activity-outcome-reconciliation-process CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-INDETERMINATE-ACTIVITY-OUTCOME-RECONCILIATION-PROCESS	Process: Indeterminate Activity Outcome Reconciliation Process (p. 993)	Proposed / Proposed
HCM-PR-operational-work-progression-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.PR-OPERATIONAL-WORK-PROGRESSION-PROCESS	Process: Operational Work Progression Process (p. 617)	Canonical / Proposed
HCM-PR-patient-transport-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.PR-PATIENT-TRANSPORT-PROCESS	Process: Patient Transport Process (p. 638)	Canonical / Proposed
HCM-PR-practitioner-verification-process CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.PR-PRACTITIONER-VERIFICATION-PROCESS	Process: Practitioner Verification Process (p. 262)	Canonical / Proposed
HCM-PR-referral-progression-process CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.PR-REFERRAL-PROGRESSION-PROCESS	Process: Referral Progression Process (p. 354)	Canonical / Proposed
HCM-PR-specimen-transport-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.PR-SPECIMEN-TRANSPORT-PROCESS	Process: Specimen Transport Process (p. 916)	Canonical / Proposed
HCM-PR-synthetic-task-progression-process CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-SYNTHETIC-TASK-PROGRESSION-PROCESS	Process: Synthetic Task Progression Process (p. 919)	Canonical / Proposed
HCM-PR-theatre-case-progression-process CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.PR-THEATRE-CASE-PROGRESSION-PROCESS	Process: Theatre Case Progression Process (p. 547)	Canonical / Proposed
HCM-PR-to-do-progression-process CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-TO-DO-PROGRESSION-PROCESS	Process: To Do Progression Process (p. 918)	Canonical / Proposed
HCM-PR-work-order-progression-process CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.PR-WORK-ORDER-PROGRESSION-PROCESS	Process: Work Order Progression Process (p. 917)	Canonical / Proposed
HCM-SV-active-problem-allergy-summary-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-ACTIVE-PROBLEM-ALLERGY-SUMMARY-QUERY	Service: Active Problem & Allergy Summary Query (p. 684)	Canonical / Proposed

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HCM-SV-activity-escalation-telemetry CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-ACTIVITY-ESCALATION-TELEMETRY	Service: Activity Escalation Telemetry (p. 909)	Canonical / Proposed
HCM-SV-acute-admission-dossier-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-ADMISSION-DOSSIER-QUERY	Service: Acute Admission Dossier Query (p. 369)	Canonical / Proposed
HCM-SV-acute-clinical-alert-dispatch CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-CLINICAL-ALERT-DISPATCH	Service: Acute Clinical Alert Dispatch (p. 370)	Canonical / Proposed
HCM-SV-addressed-document-delivery-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-ADDRESSED-DOCUMENT-DELIVERY-SERVICE	Service: Addressed Document Delivery Service (p. 699)	Canonical / Proposed
HCM-SV-allergy-adverse-reaction-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.SV-ALLERGY-ADVERSE-REACTION-QUERY	Service: Allergy & Adverse Reaction Query (p. 425)	Canonical / Proposed
HCM-SV-ambulatory-session-status-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.SV-AMBULATORY-SESSION-STATUS-QUERY	Service: Ambulatory Session Status Query (p. 568)	Canonical / Proposed
HCM-SV-appointment-notification-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-NOTIFICATION-INGRESS	Service: Appointment Notification Ingress (p. 360)	Canonical / Proposed
HCM-SV-appointment-schedule-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-SCHEDULE-QUERY	Service: Appointment Schedule Query (p. 362)	Canonical / Proposed
HCM-SV-associated-binary-content-retrieval CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.SV-ASSOCIATED-BINARY-CONTENT-RETRIEVAL	Service: Associated Binary Content Retrieval (p. 668)	Proposed / Proposed
HCM-SV-authorised-research-information-export CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.SV-AUTHORISED-RESEARCH-INFORMATION-EXPORT	Service: Authorised Research Information Export (p. 815)	Proposed / Proposed
HCM-SV-bed-availability-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-AVAILABILITY-QUERY	Service: Bed Availability Query (p. 577)	Canonical / Proposed
HCM-SV-bed-turnover-lifecycle-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-TURNOVER-LIFECYCLE-SERVICE	Service: Bed Turnover Lifecycle Service (p. 579)	Canonical / Proposed

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HCM-SV-care-place-specification-lookup CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.SV-CARE-PLACE-SPECIFICATION-LOOKUP	Service: Care-Place Specification Lookup (p. 286)	Canonical / Proposed
HCM-SV-care-plan-change-notification CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CHANGE-NOTIFICATION	Service: Care Plan Change Notification (p. 475)	Canonical / Proposed
HCM-SV-care-plan-context-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CONTEXT-QUERY	Service: Care Plan Context Query (p. 474)	Canonical / Proposed
HCM-SV-care-plan-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-INGRESS	Service: Care Plan Ingress (p. 472)	Canonical / Proposed
HCM-SV-chronic-care-timeline-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.SV-CHRONIC-CARE-TIMELINE-QUERY	Service: Chronic Care Timeline Query (p. 674)	Canonical / Proposed
HCM-SV-client-relationship-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.SV-CLIENT-RELATIONSHIP-QUERY	Service: Client Relationship Query (p. 235)	Canonical / Proposed
HCM-SV-clinic-queue-telemetry-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.SV-CLINIC-QUEUE-TELEMETRY-SERVICE	Service: Clinic Queue Telemetry Service (p. 522)	Canonical / Proposed
HCM-SV-clinical-collaboration-space-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-CLINICAL-COLLABORATION-SPACE-SERVICE	Service: Clinical Collaboration Space Service (p. 764)	Canonical / Proposed
HCM-SV-clinical-document-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-INGRESS	Service: Clinical Document Ingress (p. 450)	Canonical / Proposed
HCM-SV-clinical-document-lifecycle-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-LIFECYCLE-SERVICE	Service: Clinical Document Lifecycle Service (p. 452)	Canonical / Proposed
HCM-SV-clinical-event-syndication-stream CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-CLINICAL-EVENT-SYNDICATION-STREAM	Service: Clinical Event Syndication Stream (p. 700)	Canonical / Proposed
HCM-SV-clinical-information-search-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-INFORMATION-SEARCH-SERVICE	Service: Clinical Information Search Service (p. 707)	Canonical / Proposed
HCM-SV-clinical-privilege-verification CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-CLINICAL-PRIVILEGE-VERIFICATION	Service: Clinical Privilege Verification (p. 262)	Canonical / Proposed

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HCM-SV-clinical-resource-availability-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.SV-CLINICAL-RESOURCE-AVAILABILITY-QUERY	Service: Clinical Resource Availability Query (p. 315)	Canonical / Proposed
HCM-SV-clinical-resource-retrieval-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-RESOURCE-RETRIEVAL-SERVICE	Service: Clinical Resource Retrieval Service (p. 709)	Canonical / Proposed
HCM-SV-collaboration-clinical-summary-resolution CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-CLINICAL-SUMMARY-RESOLUTION	Service: Collaboration Clinical Summary Resolution (p. 766)	Canonical / Proposed
HCM-SV-collaboration-space-governance-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-SPACE-GOVERNANCE-SERVICE	Service: Collaboration Space Governance Service (p. 767)	Canonical / Proposed
HCM-SV-communicate-service-assurance-outcome CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.SV-COMMUNICATE-SERVICE-ASSURANCE-OUTCOME	Service: Communicate Service Assurance Outcome (p. 915)	Canonical / Proposed
HCM-SV-communication-audit-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-AUDIT-QUERY	Service: Communication Audit Query (p. 486)	Canonical / Proposed
HCM-SV-communication-delivery-status-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-DELIVERY-STATUS-QUERY	Service: Communication Delivery Status Query (p. 485)	Canonical / Proposed
HCM-SV-community-care-encounter-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-ENCOUNTER-INGRESS	Service: Community Care Encounter Ingress (p. 464)	Canonical / Proposed
HCM-SV-community-care-team-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-TEAM-QUERY	Service: Community Care Team Query (p. 466)	Canonical / Proposed
HCM-SV-confidentiality-restriction-evaluation CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.SV-CONFIDENTIALITY-RESTRICTION-EVALUATION	Service: Confidentiality Restriction Evaluation (p. 242)	Canonical / Proposed
HCM-SV-consent-decision-service CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-PRIVACY.SV-CONSENT-DECISION-SERVICE	Service: Consent Decision Service (p. 241)	Canonical / Proposed

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HCM-SV-consent-enforcement-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-CONSENT-ENFORCEMENT-SERVICE	Service: Consent Enforcement Service (p. 724)	Canonical / Proposed
HCM-SV-contextual-presentation-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.SV-CONTEXTUAL-PRESENTATION-SERVICE	Service: Contextual Presentation Service (p. 791)	Canonical / Proposed
HCM-SV-demographic-change-distribution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.SV-DEMOGRAPHIC-CHANGE-DISTRIBUTION	Service: Demographic Change Distribution (p. 231)	Canonical / Proposed
HCM-SV-demographic-intake-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-INTAKE-SERVICE	Service: Demographic Intake Service (p. 340)	Canonical / Proposed
HCM-SV-demographic-update-notification CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-UPDATE-NOTIFICATION	Service: Demographic Update Notification (p. 342)	Canonical / Proposed
HCM-SV-device-association-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-ASSOCIATION-QUERY	Service: Device Association Query (p. 323)	Canonical / Proposed
HCM-SV-device-definition-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-DEFINITION-QUERY	Service: Device Definition Query (p. 320)	Canonical / Proposed
HCM-SV-device-endpoint-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-ENDPOINT-RESOLUTION	Service: Device Endpoint Resolution (p. 324)	Canonical / Proposed
HCM-SV-device-instance-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.SV-DEVICE-INSTANCE-RESOLUTION	Service: Device Instance Resolution (p. 322)	Canonical / Proposed
HCM-SV-diagnostic-correlation-resolution CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-CORRELATION-RESOLUTION	Service: Diagnostic Correlation Resolution (p. 414)	Canonical / Proposed
HCM-SV-diagnostic-publication-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-DIAGNOSTIC-PUBLICATION-SERVICE	Service: Diagnostic Publication Service (p. 407)	Canonical / Proposed
HCM-SV-diagnostic-report-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-REPORT-INGRESS	Service: Diagnostic Report Ingress (p. 415)	Canonical / Proposed
HCM-SV-diagnostic-result-delivery-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-RESULT-DELIVERY-SERVICE	Service: Diagnostic Result Delivery Service (p. 417)	Canonical / Proposed
HCM-SV-discharge-dossier-packaging-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.SV-DISCHARGE-DOSSIER-PACKAGING-SERVICE	Service: Discharge Dossier Packaging Service (p. 535)	Canonical / Proposed

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HCM-SV-discharge-readiness-telemetry CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.SV-DISCHARGE-READINESS-TELEMETRY	Service: Discharge Readiness Telemetry (p. 652)	Canonical / Proposed
HCM-SV-dispense-event-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-DISPENSE-EVENT-INGRESS	Service: Dispense Event Ingress (p. 433)	Canonical / Proposed
HCM-SV-document-metadata-registry-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-DOCUMENT-METADATA-REGISTRY-QUERY	Service: Document Metadata Registry Query (p. 453)	Canonical / Proposed
HCM-SV-ed-operational-status-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.SV-ED-OPERATIONAL-STATUS-QUERY	Service: ED Operational Status Query (p. 561)	Canonical / Proposed
HCM-SV-ed-triage-stream-ingress CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.SV-ED-TRIAGE-STREAM-INGRESS	Service: ED Triage Stream Ingress (p. 558)	Canonical / Proposed
HCM-SV-emergency-fast-path-lhr-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.SV-EMERGENCY-FAST-PATH-LHR-QUERY	Service: Emergency Fast-Path LHR Query (p. 559)	Canonical / Proposed
HCM-SV-encounter-context-resolution CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-CONTEXT-RESOLUTION	Service: Encounter Context Resolution (p. 378)	Canonical / Proposed
HCM-SV-encounter-lifecycle-event-notification CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LIFECYCLE-EVENT-NOTIFICATION	Service: Encounter Lifecycle Event Notification (p. 379)	Canonical / Proposed
HCM-SV-encounter-location-history-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LOCATION-HISTORY-QUERY	Service: Encounter Location History Query (p. 381)	Canonical / Proposed
HCM-SV-entity-operational-input-submission CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-ENTITY-SPECIFIC-OPERATIONAL-COORDINATION.SV-ENTITY-OPERATIONAL-INPUT-SUBMISSION	Service: Entity Operational Input Submission (p. 987)	Proposed / Proposed
HCM-SV-exchange-profile-specification-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-EXCHANGE-PROFILE-SPECIFICATION-QUERY	Service: Exchange Profile Specification Query (p. 808)	Canonical / Proposed
HCM-SV-federated-health-information-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-FEDERATED-HEALTH-INFORMATION-QUERY	Service: Federated Health Information Query (p. 698)	Canonical / Proposed

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HCM-SV-governed-reference-dataset-content-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.SV-GOVERNED-REFERENCE-DATASET-CONTENT-QUERY	Service: Governed Reference Dataset Content Query (p. 752)	Proposed / Proposed
HCM-SV-healthcare-service-directory-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-HEALTHCARE-SERVICE-DIRECTORY-QUERY	Service: Healthcare Service Directory Query (p. 295)	Canonical / Proposed
HCM-SV-healthcare-subject-context-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-HEALTHCARE-SUBJECT.SV-HEALTHCARE-SUBJECT-CONTEXT-RESOLUTION	Service: Healthcare Subject Context Resolution (p. 230)	Canonical / Proposed
HCM-SV-hie-submission-gateway-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-HIE-SUBMISSION-GATEWAY-SERVICE	Service: HIE Submission Gateway Service (p. 696)	Canonical / Proposed
HCM-SV-historical-clinical-record-access CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-HISTORICAL-CLINICAL-RECORD-ACCESS	Service: Historical Clinical Record Access (p. 685)	Canonical / Proposed
HCM-SV-immunisation-history-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.SV-IMMUNISATION-HISTORY-QUERY	Service: Immunisation History Query (p. 334)	Canonical / Proposed
HCM-SV-information-dictionary-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-INFORMATION-DICTIONARY-QUERY	Service: Information Dictionary Query (p. 805)	Canonical / Proposed
HCM-SV-location-directory-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.SV-LOCATION-DIRECTORY-QUERY	Service: Location Directory Query (p. 284)	Canonical / Proposed
HCM-SV-location-hierarchy-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.SV-LOCATION-HIERARCHY-RESOLUTION	Service: Location Hierarchy Resolution (p. 285)	Canonical / Proposed
HCM-SV-longitudinal-clinical-record-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-LONGITUDINAL-CLINICAL-RECORD-QUERY	Service: Longitudinal Clinical Record Query (p. 682)	Canonical / Proposed
HCM-SV-longitudinal-medication-history-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.SV-LONGITUDINAL-MEDICATION-HISTORY-QUERY	Service: Longitudinal Medication History Query (p. 424)	Canonical / Proposed
HCM-SV-mandatory-audit-evidence-capture CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-SIGNIFICANT-ACTIVITY-AUDIT-CAPTURE.SV-MANDATORY-AUDIT-EVIDENCE-CAPTURE	Service: Mandatory Audit Evidence Capture (p. 985)	Proposed / Proposed

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HCM-SV-medication-administration-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ADMINISTRATION-INGRESS	Service: Medication Administration Ingress (p. 434)	Canonical / Proposed
HCM-SV-medication-order-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ORDER-INGRESS	Service: Medication Order Ingress (p. 431)	Canonical / Proposed
HCM-SV-modality-worklist-distribution CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-MODALITY-WORKLIST-DISTRIBUTION	Service: Modality Worklist Distribution (p. 405)	Canonical / Proposed
HCM-SV-on-call-specialist-lookup CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.SV-ON-CALL-SPECIALIST-LOOKUP	Service: On-Call Specialist Lookup (p. 606)	Canonical / Proposed
HCM-SV-operational-collaboration-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.SV-OPERATIONAL-COLLABORATION-SERVICE	Service: Operational Collaboration Service (p. 772)	Canonical / Proposed
HCM-SV-operational-credential-checkpoint-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CREDENTIAL-MANAGEMENT.SV-OPERATIONAL-CREDENTIAL-CHECKPOINT-SERVICE	Service: Operational Credential Checkpoint Service (p. 901)	Canonical / Proposed
HCM-SV-operational-work-status-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.SV-OPERATIONAL-WORK-STATUS-QUERY	Service: Operational Work Status Query (p. 616)	Canonical / Proposed
HCM-SV-order-cancellation-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-CANCELLATION-SERVICE	Service: Order Cancellation Service (p. 396)	Canonical / Proposed
HCM-SV-order-dispatch-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-DISPATCH-SERVICE	Service: Order Dispatch Service (p. 393)	Canonical / Proposed
HCM-SV-order-outcome-notification CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-OUTCOME-NOTIFICATION	Service: Order Outcome Notification (p. 397)	Canonical / Proposed
HCM-SV-order-requisition-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-REQUISITION-INGRESS	Service: Order Requisition Ingress (p. 391)	Canonical / Proposed
HCM-SV-order-status-tracking-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-STATUS-TRACKING-QUERY	Service: Order Status & Tracking Query (p. 394)	Canonical / Proposed
HCM-SV-organisation-contact-lookup CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATION-CONTACT-LOOKUP	Service: Organisation Contact Lookup (p. 275)	Canonical / Proposed
HCM-SV-organisation-directory-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATION-DIRECTORY-QUERY	Service: Organisation Directory Query (p. 272)	Canonical / Proposed

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HCM-SV-organisation-verification-service CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATION-VERIFICATION-SERVICE	Service: Organisation Verification Service (p. 271)	Canonical / Proposed
HCM-SV-organisational-hierarchy-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.SV-ORGANISATIONAL-HIERARCHY-RESOLUTION	Service: Organisational Hierarchy Resolution (p. 274)	Canonical / Proposed
HCM-SV-outreach-batch-reconciliation-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.SV-OUTREACH-BATCH-RECONCILIATION-SERVICE	Service: Outreach Batch Reconciliation Service (p. 471)	Canonical / Proposed
HCM-SV-patient-transport-tracking-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.SV-PATIENT-TRANSPORT-TRACKING-SERVICE	Service: Patient Transport Tracking Service (p. 638)	Canonical / Proposed
HCM-SV-person-identifier-correlation CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.SV-PERSON-IDENTIFIER-CORRELATION	Service: Person Identifier Correlation (p. 221)	Canonical / Proposed
HCM-SV-person-identifier-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.SV-PERSON-IDENTIFIER-RESOLUTION	Service: Person Identifier Resolution (p. 220)	Canonical / Proposed
HCM-SV-person-identity-correction CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-PERSON-IDENTITY.SV-PERSON-IDENTITY-CORRECTION	Service: Person Identity Correction (p. 223)	Canonical / Proposed
HCM-SV-policy-evaluation-authorisation-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-POLICY-EVALUATION-AUTHORISATION-SERVICE	Service: Policy Evaluation & Authorisation Service (p. 723)	Canonical / Proposed
HCM-SV-practitioner-affiliation-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-AFFILIATION-RESOLUTION	Service: Practitioner Affiliation Resolution (p. 260)	Canonical / Proposed
HCM-SV-practitioner-directory-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-DIRECTORY-QUERY	Service: Practitioner Directory Query (p. 256)	Canonical / Proposed
HCM-SV-practitioner-endpoint-lookup CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-ENDPOINT-LOOKUP	Service: Practitioner Endpoint Lookup (p. 259)	Canonical / Proposed
HCM-SV-practitioner-role-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-ROLE-RESOLUTION	Service: Practitioner Role Resolution (p. 258)	Canonical / Proposed

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HCM-SV-practitioner-verification-service CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.SV-PRACTITIONER-VERIFICATION-SERVICE	Service: Practitioner Verification Service (p. 255)	Canonical / Proposed
HCM-SV-preventive-screening-status-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.SV-PREVENTIVE-SCREENING-STATUS-QUERY	Service: Preventive Screening Status Query (p. 335)	Canonical / Proposed
HCM-SV-primary-care-summary-ingress CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.SV-PRIMARY-CARE-SUMMARY-INGRESS	Service: Primary Care Summary Ingress (p. 673)	Canonical / Proposed
HCM-SV-procedural-documentation-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURAL-DOCUMENTATION-INGRESS	Service: Procedural Documentation Ingress (p. 443)	Canonical / Proposed
HCM-SV-procedure-booking-ingress CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURE-BOOKING-INGRESS	Service: Procedure Booking Ingress (p. 441)	Canonical / Proposed
HCM-SV-referral-context-query CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-CONTEXT-QUERY	Service: Referral Context Query (p. 352)	Canonical / Proposed
HCM-SV-referral-status-outcome-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-STATUS-OUTCOME-SERVICE	Service: Referral Status & Outcome Service (p. 353)	Canonical / Proposed
HCM-SV-referral-submission-service CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-SUBMISSION-SERVICE	Service: Referral Submission Service (p. 350)	Canonical / Proposed
HCM-SV-regional-health-system-capacity-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.SV-REGIONAL-HEALTH-SYSTEM-CAPACITY-QUERY	Service: Regional Health System Capacity Query (p. 585)	Canonical / Proposed
HCM-SV-remote-telemetry-ingress CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-REMOTE-TELEMETRY-INGRESS	Service: Remote Telemetry Ingress (p. 681)	Canonical / Proposed
HCM-SV-representative-authority-verification CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-ADMINISTRATION.CT3-BEC-CLIENT-RELATIONSHIP.SV-REPRESENTATIVE-AUTHORITY-VERIFICATION	Service: Representative Authority Verification (p. 236)	Canonical / Proposed
HCM-SV-request-service-assurance CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.SV-REQUEST-SERVICE-ASSURANCE	Service: Request Service Assurance (p. 913)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-SV-request-service-assurance-status CT1-BEC-INDEPENDENT-SERVICE-ASSURANCE-ENABLEMENT.CT2-BEC-INDEPENDENT-ASSURANCE-EVALUATION-ENABLEMENT.CT3-BEC-GOVERNED-ASSURANCE.SV-REQUEST-SERVICE-ASSURANCE-STATUS	Service: Request Service Assurance Status (p. 914)	Canonical / Proposed
HCM-SV-secure-clinical-message-dispatch CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-SECURE-CLINICAL-MESSAGE-DISPATCH	Service: Secure Clinical Message Dispatch (p. 483)	Canonical / Proposed
HCM-SV-security-audit-ingress CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-AUDIT-INGRESS	Service: Security Audit Ingress (p. 727)	Canonical / Proposed
HCM-SV-security-context-validation-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-CONTEXT-VALIDATION-SERVICE	Service: Security Context Validation Service (p. 726)	Canonical / Proposed
HCM-SV-semantic-conformance-verification-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-CONFORMANCE-VERIFICATION-SERVICE	Service: Semantic Conformance Verification Service (p. 809)	Canonical / Proposed
HCM-SV-semantic-model-definition-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-MODEL-DEFINITION-SERVICE	Service: Semantic Model Definition Service (p. 804)	Canonical / Proposed
HCM-SV-service-availability-query CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-SERVICE-AVAILABILITY-QUERY	Service: Service Availability Query (p. 298)	Canonical / Proposed
HCM-SV-service-eligibility-evaluation CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-SERVICE-ELIGIBILITY-EVALUATION	Service: Service Eligibility Evaluation (p. 299)	Canonical / Proposed
HCM-SV-service-offering-readiness-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-OFFERING-READINESS-COORDINATION.SV-SERVICE-OFFERING-READINESS-QUERY	Service: Service Offering Readiness Query (p. 995)	Proposed / Proposed
HCM-SV-service-provision-resolution CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.SV-SERVICE-PROVISION-RESOLUTION	Service: Service Provision Resolution (p. 296)	Canonical / Proposed
HCM-SV-specimen-transit-tracking-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.SV-SPECIMEN-TRANSIT-TRACKING-SERVICE	Service: Specimen Transit Tracking Service (p. 644)	Canonical / Proposed
HCM-SV-staff-presence-telemetry-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.SV-STAFF-PRESENCE-TELEMETRY-QUERY	Service: Staff Presence Telemetry Query (p. 599)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-SV-standards-communication-gateway-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-COMMUNICATION.SV-STANDARDS-COMMUNICATION-GATEWAY-SERVICE	Service: Standards Communication Gateway Service (p. 903)	Canonical / Proposed
HCM-SV-synthetic-task-execution-service CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-SYNTHETIC-TASK-EXECUTION-SERVICE	Service: Synthetic Task Execution Service (p. 907)	Canonical / Proposed
HCM-SV-temporal-context-resolution CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.SV-TEMPORAL-CONTEXT-RESOLUTION	Service: Temporal Context Resolution (p. 785)	Canonical / Proposed
HCM-SV-terminology-binding-specification-lookup CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-TERMINOLOGY-BINDING-SPECIFICATION-LOOKUP	Service: Terminology Binding Specification Lookup (p. 806)	Canonical / Proposed
HCM-SV-terminology-concept-resolution-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-CONCEPT-RESOLUTION-SERVICE	Service: Terminology Concept Resolution Service (p. 754)	Canonical / Proposed
HCM-SV-terminology-mapping-service CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-MAPPING-SERVICE	Service: Terminology Mapping Service (p. 755)	Canonical / Proposed
HCM-SV-theatre-case-progression-telemetry CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.SV-THEATRE-CASE-PROGRESSION-TELEMETRY	Service: Theatre Case Progression Telemetry (p. 546)	Canonical / Proposed
HCM-SV-to-do-management-service CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-TO-DO-MANAGEMENT-SERVICE	Service: To Do Management Service (p. 906)	Canonical / Proposed
HCM-SV-unified-calendar-projection-query CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.SV-UNIFIED-CALENDAR-PROJECTION-QUERY	Service: Unified Calendar Projection Query (p. 784)	Canonical / Proposed
HCM-SV-virtual-encounter-context-resolution CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-VIRTUAL-ENCOUNTER-CONTEXT-RESOLUTION	Service: Virtual Encounter Context Resolution (p. 679)	Canonical / Proposed
HCM-SV-ward-operational-summary-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.SV-WARD-OPERATIONAL-SUMMARY-QUERY	Service: Ward Operational Summary Query (p. 537)	Canonical / Proposed

Reference / structural identity	Element / page	Element / path standing
HCM-SV-ward-round-clinical-briefing-query CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.SV-WARD-ROUND-CLINICAL-BRIEFING-QUERY	Service: Ward Round Clinical Briefing Query (p. 534)	Canonical / Proposed
HCM-SV-work-dispatch-service CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.SV-WORK-DISPATCH-SERVICE	Service: Work Dispatch Service (p. 614)	Canonical / Proposed
HCM-SV-work-order-coordination-service CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-WORKFLOW-ACTIVITY-COORDINATION.SV-WORK-ORDER-COORDINATION-SERVICE	Service: Work Order Coordination Service (p. 905)	Canonical / Proposed

34.1. Historical allocated IDs

Historical ID alias	Current identity	Current structural ID
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-RELATIONSHIP.FEAT-EM-15	Feature: Practitioner-Organisation Affiliation (p. 252)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PRACTITIONER-ROLE.FEAT-EM-15
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-PROVIDER-ADMINISTRATION.CT3-BEC-PROVIDER-ENGAGEMENT.FEAT-EM-16	Feature: Clinical Privileging Record (p. 625)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.CT3-BEC-PROVIDER-CLINICAL-QUALIFICATION-TRACKING.FEAT-EM-16
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATIONAL-STRUCTURE.FEAT-EM-19	Feature: Hierarchical Structure Navigation (p. 268)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-19
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-ORGANISATION-CONTACT.FEAT-EM-20	Feature: Directory Contact Management (p. 269)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-ORGANISATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-ORGANISATION.FEAT-EM-20

Historical ID alias	Current identity	Current structural ID
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-LOCATION-STRUCTURE.FEAT-EM-22	Feature: Physical Hierarchy Modeling (p. 280)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-LOCATION-ADMINISTRATION.CT3-BEC-HEALTHCARE-LOCATION.FEAT-EM-22
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-PROVISION-RELATIONSHIP.FEAT-EM-25	Feature: Service-Location-Provider Binding (p. 291)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-25
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-AVAILABILITY-DEFINITION.FEAT-EM-26	Feature: Operating Schedule Specification (p. 292)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-26
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-SERVICE-ELIGIBILITY-DEFINITION.FEAT-EM-27	Feature: Eligibility & Catchment Specification (p. 293)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-SERVICE-ADMINISTRATION.CT3-BEC-HEALTHCARE-SERVICE-DEFINITION.FEAT-EM-27
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-INSTANCE.FEAT-EM-29	Feature: Device Serial Tracking (p. 310)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-DEFINITION.FEAT-EM-29
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-ASSOCIATION.FEAT-EM-30	Feature: Device-Patient/Location Assignment (p. 312)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-EM-30

Historical ID alias	Current identity	Current structural ID
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-CONNECTIVITY.FEAT-EM-31	Feature: Device Endpoint Registration (p. 318)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-OA-M-METADATA-MANAGEMENT.FEAT-EM-31
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION	BEC CT2: Client-Centred Administration (p. 328)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION	BEC CT2: Referral Administration (p. 342)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION	BEC CT2: Scheduling Administration (p. 355)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION	BEC CT2: Episode & Encounter Administration (p. 363)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION	BEC CT2: Order Administration (p. 383)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION	BEC CT2: Diagnostic Administration (p. 399)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION	BEC CT2: Medication Administration (p. 417)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION	BEC CT2: Procedure Administration (p. 435)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION	BEC CT2: Clinical Record Administration (p. 443)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION	BEC CT2: Care Plan Administration (p. 455)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION	BEC CT2: Clinical Communication Administration (p. 476)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FINANCIAL-INFORMATION-ENABLEMENT.CT3-BEC-FUNDING-COVERAGE-ADMINISTRATION	BEC CT2: Funding & Coverage Administration (p. 487)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-FUNDING-COVERAGE-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FINANCIAL-INFORMATION-ENABLEMENT.CT3-BEC-CLINICAL-CHARGING-CLAIMS-ADMINISTRATION	BEC CT2: Clinical Charging & Claims Administration (p. 489)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-CHARGING-CLAIMS-ADMINISTRATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FEAT-SA-01	Feature: Demographic Update Ingestion (p. 337)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE.FEAT-SA-01
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FEAT-SA-02	Feature: Demographic Change Distribution (p. 338)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-CLIENT-INTAKE-DEMOGRAPHIC-UPDATE.FEAT-SA-02
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-SA-03	Feature: Referral Document Ingestion (p. 346)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-03

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-SA-04	Feature: Referral Supporting Information (p. 346)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-04
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-SA-05	Feature: Referral Status & Outcome Tracking (p. 347)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-SA-05
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FEAT-SA-06	Feature: Appointment Notification Ingestion (p. 358)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION.FEAT-SA-06
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FEAT-SA-07	Feature: Appointment Status Synchronization (p. 359)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.CT3-BEC-APPOINTMENT-BOOKING-ADMINISTRATION.FEAT-SA-07
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-SA-08	Feature: Encounter Context Creation (p. 373)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-08
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-SA-09	Feature: Encounter State Progression (p. 374)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-09

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-SA-10	Feature: Encounter Bed/Location Association (p. 375)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-SA-10
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-11	Feature: Order Request Ingestion (p. 386)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-11
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-12	Feature: Order Destination Resolution & Routing (p. 387)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-12
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-13	Feature: Order Closed-Loop Progression Tracking (p. 388)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-13
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-SA-14	Feature: Order Cancellation & Modification Coordination (p. 389)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-SA-14
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-SA-15	Feature: Diagnostic Request Correlation (p. 409)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-15

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-SA-16	Feature: Diagnostic Report Ingestion & Binding (p. 410)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-16
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-SA-17	Feature: Diagnostic Report Distribution (p. 411)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-SA-17
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FEAT-SA-18	Feature: Medication Order Ingestion (p. 428)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-18
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FEAT-SA-19	Feature: Dispense Event Tracking (p. 429)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-19
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FEAT-SA-20	Feature: Medication Administration Record (MAR) Ingestion (p. 430)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-ORDER-DISPENSE-MANAGEMENT.FEAT-SA-20

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FEAT-SA-21	Feature: Procedure Booking Ingestion (p. 438)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION.FEAT-SA-21
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FEAT-SA-22	Feature: Procedural Documentation Ingestion (p. 439)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.CT3-BEC-PROCEDURE-SURGICAL-ADMINISTRATION.FEAT-SA-22
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FEAT-SA-23	Feature: Clinical Document Ingestion (p. 447)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-23
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FEAT-SA-24	Feature: Document Versioning & Supersession (p. 448)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-24
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FEAT-SA-25	Feature: Document Metadata Indexing (p. 449)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.CT3-BEC-CLINICAL-DOCUMENTATION-LIFECYCLE-ADMINISTRATION.FEAT-SA-25

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FEAT-SA-26	Feature: Care Plan Ingestion & Distribution (p. 458)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION.FEAT-SA-26
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FEAT-SA-27	Feature: Clinical Communication Distribution (p. 479)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-27
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FEAT-SA-28	Feature: Delivery Acknowledgement Tracking (p. 480)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-28
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FEAT-SA-29	Feature: Communication Audit Logging (p. 481)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.CT3-BEC-SECURE-CLINICAL-COMMUNICATION.FEAT-SA-29
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-CLINICAL-ASSESSMENT	BEC CT2: Clinical Assessment (p. 492)	CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-CLINICAL-ASSESSMENT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE	BEC CT3: Primary Care Coordination (p. 669)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE	BEC CT3: Acute Care Monitoring & Event Coordination (p. 365)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE	BEC CT3: Emergency Care Progression (p. 554)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE	BEC CT3: Multidisciplinary Inpatient Care Coordination (p. 529)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-AMBULATORY-CARE	BEC CT2: Ambulatory Care (p. 498)	CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-AMBULATORY-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-SURGICAL-PROCEDURAL-CARE	BEC CT2: Surgical & Procedural Care (p. 499)	CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-SURGICAL-PROCEDURAL-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES	BEC CT3: Diagnostic Service Integration (p. 401)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-THERAPEUTIC-SERVICES	BEC CT2: Therapeutic Services (p. 501)	CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-THERAPEUTIC-SERVICES
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY	BEC CT3: Medication Therapy Provision and Monitoring (p. 419)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-ALLIED-HEALTH	BEC CT2: Allied Health (p. 504)	CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-ALLIED-HEALTH
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE	BEC CT3: Screening and Immunisation Enablement (p. 329)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-REHABILITATION	BEC CT2: Rehabilitation (p. 506)	CT1-BEC-CLINICAL-SERVICE-DEPENDENCY-MANAGEMENT.CT2-BEC-REHABILITATION
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE	BEC CT3: Community Care Coordination (p. 460)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE	BEC CT3: Outreach Care Continuity (p. 466)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE	BEC CT3: Remote Patient Monitoring (p. 675)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FEAT-SD-01	Feature: General Practice Shared Record Ingestion (p. 671)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FEAT-SD-01
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FEAT-SD-02	Feature: Primary Care Notification Dispatch (p. 671)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FEAT-SD-02
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-03	Feature: Acute Clinical State Event Capture (p. 366)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-03
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-04	Feature: Acute Care Clinical Context Provision (p. 367)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FEAT-SD-04

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CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FEAT-SD-05	Feature: Emergency Department Arrival & Triage Ingestion (p. 555)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FEAT-SD-05
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FEAT-SD-06	Feature: Emergency Care Cross-Facility Correlation (p. 556)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FEAT-SD-06
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FEAT-SD-07	Feature: Inpatient Rounding Context Provision (p. 531)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FEAT-SD-07
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FEAT-SD-08	Feature: Inpatient Clinical Progression Tracking (p. 532)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FEAT-SD-08
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-09	Feature: Laboratory & Imaging Result Distribution (p. 403)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-09
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-10	Feature: Diagnostic History Consolidation (p. 404)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FEAT-SD-10

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CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-11	Feature: Medication History Reconciliation Ingestion (p. 421)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-11
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-12	Feature: Medication Administration Tracking (p. 422)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FEAT-SD-12
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-13	Feature: Immunisation Event Submission (p. 331)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-13
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-14	Feature: Screening Recall Notification Distribution (p. 332)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FEAT-SD-14
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-15	Feature: Community Encounter Ingestion (p. 462)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-15
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-16	Feature: Community Care Plan Synchronization (p. 463)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FEAT-SD-16

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CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-17	Feature: Outreach Visit Context Provision (p. 468)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-17
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-18	Feature: Outreach Encounter Outcome Ingestion (p. 469)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FEAT-SD-18
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-19	Feature: Remote Telemetry Ingestion (p. 677)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-19
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-20	Feature: Virtual Care Session Context Binding (p. 678)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FEAT-SD-20
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS	BEC CT2: Clinic & Practice Operations (p. 514)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS	BEC CT2: Ward Operations (p. 524)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS	BEC CT2: Theatre Operations (p. 538)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS	BEC CT2: Emergency Department Operations (p. 548)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS	BEC CT2: Outpatient Operations (p. 561)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT	BEC CT2: Bed & Care-Place Management (p. 569)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT	BEC CT2: Service Capacity Management (p. 581)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-CLINICAL-WORKFORCE-MANAGEMENT	BEC CT2: Clinical Workforce Capability Monitoring (p. 587)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-WORKFORCE-MANAGEMENT

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-CLINICAL-ROSTERING	BEC CT2: Clinical Rostering (p. 589)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-ROSTERING
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT	BEC CT2: Mobile Staff Management (p. 591)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT	BEC CT2: On-Call Management (p. 601)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH	BEC CT2: Work Allocation & Dispatch (p. 607)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-TRAINING-EDUCATION	BEC CT2: Clinical Training & Education (p. 618)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-TRAINING-EDUCATION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-COMPETENCY-MANAGEMENT	BEC CT2: Clinical Competency Administration (p. 619)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-COMPETENCY-MANAGEMENT

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-QUALIFICATION-MANAGEMENT	BEC CT2: Clinical Qualification Administration (p. 621)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-SUPERVISION	BEC CT2: Clinical Supervision (p. 630)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-SUPERVISION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT	BEC CT2: Patient Transport (p. 632)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION	BEC CT2: Clinical Logistics Coordination (p. 639)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT	BEC CT2: Discharge Management (p. 645)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FEAT-HSO-01	Feature: Clinic Session Tracking (p. 517)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-HSO-01

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FEAT-HSO-02	Feature: Operational Queue Progression (p. 518)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-HSO-02
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FEAT-HSO-03	Feature: Ward Patient Flow Coordination (p. 527)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-HSO-03
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FEAT-HSO-04	Feature: Clinical Handover Context Assembly (p. 528)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-HSO-04
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FEAT-HSO-05	Feature: Operating Theatre Case Progression (p. 541)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-HSO-05
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FEAT-HSO-06	Feature: Perioperative Resource Notification (p. 542)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-HSO-06
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FEAT-HSO-07	Feature: ED Bed & Bay Tracking (p. 551)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-HSO-07

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FEAT-HSO-08	Feature: ED Length of Stay Tracking (p. 552)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-HSO-08
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FEAT-HSO-09	Feature: Outpatient Arrival Registration (p. 564)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-HSO-09
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FEAT-HSO-10	Feature: Post-Clinic Order Coordination (p. 565)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-HSO-10
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FEAT-HSO-11	Feature: Bed Availability Tracking (p. 572)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-STATE-ADMINISTRATION.FEAT-HSO-11
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FEAT-HSO-12	Feature: Bed State Progression (p. 575)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION.FEAT-HSO-12
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FEAT-HSO-13	Feature: Bed Readiness Notification (p. 576)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.CT3-BEC-BED-CARE-PLACE-TURNOVER-COORDINATION.FEAT-HSO-13

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT.FEAT-HSO-14	Feature: Mobile Clinical Equipment Tracking (p. 313)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-HSO-14
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT.FEAT-HSO-15	Feature: Operational Capacity Metric Aggregation (p. 584)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.CT3-BEC-SERVICE-DEMAND-CAPACITY-COORDINATION.FEAT-HSO-15
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FEAT-HSO-16	Feature: Mobile Worker Task Dispatch (p. 594)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-HSO-16
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FEAT-HSO-17	Feature: Mobile Visit Status Tracking (p. 595)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-HSO-17
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT.FEAT-HSO-18	Feature: Active On-Call Provider Resolution (p. 604)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.CT3-BEC-ON-CALL-AVAILABILITY-CONTACT-RESOLUTION.FEAT-HSO-18
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-19	Feature: Work Item Instantiation (p. 610)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-19

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-20	Feature: Worker Matching & Allocation (p. 611)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-20
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-21	Feature: Work Dispatch Delivery (p. 612)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-21
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FEAT-HSO-22	Feature: Work Progress Oversight & Escalation (p. 613)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.CT3-BEC-OPERATIONAL-WORK-ASSIGNMENT-DISPATCH-COORDINATION.FEAT-HSO-22
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FEAT-HSO-23	Feature: Operational Credential Check (p. 626)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.CT3-BEC-PROVIDER-CLINICAL-QUALIFICATION-TRACKING.FEAT-HSO-23
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.FEAT-HSO-24	Feature: Transport Request Ingestion (p. 635)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION.FEAT-HSO-24
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.FEAT-HSO-25	Feature: Transport Dispatch & Progress Tracking (p. 636)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.CT3-BEC-PATIENT-TRANSFER-COORDINATION.FEAT-HSO-25

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.FEAT-HSO-26	Feature: Pathology Specimen Transport Tracking (p. 643)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.CT3-BEC-SPECIMEN-MOVEMENT-TRACEABILITY.FEAT-HSO-26
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FEAT-HSO-27	Feature: Discharge Milestone Tracking (p. 648)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-27
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FEAT-HSO-28	Feature: Discharge Readiness & Coordination Oversight (p. 649)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-28
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FEAT-HSO-29	Feature: Discharge Notification Dispatch (p. 650)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.CT3-BEC-DISCHARGE-PROGRESSION-COORDINATION.FEAT-HSO-29
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD	BEC CT2: Patient Clinical Record (p. 656)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT	BEC CT2: Health Information Exchange Enablement (p. 686)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS	BEC CT2: Health Information Access (p. 701)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL	BEC CT2: Health Information Control (p. 715)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES	BEC CT2: Clinical Knowledge Services (p. 742)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION	BEC CT2: Clinical Collaboration (p. 756)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION	BEC CT2: General Collaboration (p. 768)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT	BEC CT2: Calendar Management (p. 779)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT

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CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES	BEC CT2: Presentation Services (p. 786)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE	BEC CT2: Information Design Governance (p. 792)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-ISE-01	Feature: Longitudinal Health Record (LHR) Assembly (p. 659)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-01
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-ISE-02	Feature: Active Clinical Record Access (p. 660)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-02
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-ISE-03	Feature: Durable Clinical Record Preservation (p. 661)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-ISE-03

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-04	Feature: Direct Submission Ingestion & Delivery (p. 690)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-04
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-05	Feature: Clinical Record Retrieval Mediation (p. 691)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-05
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-06	Feature: Addressed Clinical Distribution (p. 692)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-06
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FEAT-ISE-07	Feature: Event-Driven Syndication (p. 693)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.CT3-BEC-HIE-IMPLEMENTATION-MANAGEMENT.FEAT-ISE-07
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FEAT-ISE-08	Feature: Federated Clinical Query (p. 704)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.CT3-BEC-GOVERNED-HEALTH-INFORMATION-RETRIEVAL.FEAT-ISE-08

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FEAT-ISE-09	Feature: Access-Controlled Information Filtering (p. 705)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.CT3-BEC-GOVERNED-HEALTH-INFORMATION-RETRIEVAL.FEAT-ISE-09
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FEAT-ISE-11	Feature: Health Information Access & Consent Control (p. 719)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.CT3-BEC-PATIENT-HEALTH-INFORMATION-RESTRICTION-MANAGEMENT.FEAT-ISE-11
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-ISE-14	Feature: Clinical Concept Resolution (p. 745)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-14
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FEAT-ISE-14	Feature: Clinical Concept Resolution (p. 745)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-14
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-ISE-15	Feature: Bidirectional Terminology Mapping (p. 746)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-15

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FEAT-ISE-15	Feature: Bidirectional Terminology Mapping (p. 746)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-CLINICAL-KNOWLEDGE-REQUEST-ADMINISTRATION.FEAT-ISE-15
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-ISE-16	Feature: Care-Team Clinical Collaboration Coordination (p. 759)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-16
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-ISE-17	Feature: In-Conversation LHR Query Resolution (p. 760)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-17
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-ISE-18	Feature: Zero-PHI Collaboration Metadata Governance (p. 761)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-ISE-18
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION.FEAT-ISE-19	Feature: Operational Team Chat Enablement (p. 771)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.CT3-BEC-OPERATIONAL-COLLABORATION.FEAT-ISE-19
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FEAT-ISE-24	Feature: Schedule Projection Integration (p. 781)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION.FEAT-ISE-24

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CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FEAT-ISE-25	Feature: Temporal Event Correlation (p. 782)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.CT3-BEC-OPERATIONAL-CLINICAL-CALENDAR-COORDINATION.FEAT-ISE-25
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES.FEAT-ISE-26	Feature: Longitudinal Clinical & Operational Presentation (p. 789)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.CT3-BEC-CONTEXTUAL-INFORMATION-PRESENTATION.FEAT-ISE-26
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-ISE-27	Feature: Information Standards & Semantic Governance (p. 795)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-ISE-27
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-ISE-28	Feature: Semantic Conformance Verification (p. 801)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-UTILISATION-SUPPORT.FEAT-ISE-28
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-RECEIVE-CLIENT-DEMOGRAPHIC-CHANGE	Function: Receive Client Demographic Change (p. 339)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-RECEIVE-CLIENT-DEMOGRAPHIC-CHANGE

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-INTAKE-SERVICE	Service: Demographic Intake Service (p. 340)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-INTAKE-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-DISTRIBUTE-GOVERNED-DEMOGRAPHIC-CHANGE	Function: Distribute Governed Demographic Change (p. 341)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.FN-DISTRIBUTE-GOVERNED-DEMOGRAPHIC-CHANGE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-UPDATE-NOTIFICATION	Service: Demographic Update Notification (p. 342)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.SV-DEMOGRAPHIC-UPDATE-NOTIFICATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FN-RECEIVE-REFERRAL	Function: Receive Referral (p. 349)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-RECEIVE-REFERRAL
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-SUBMISSION-SERVICE	Service: Referral Submission Service (p. 350)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-SUBMISSION-SERVICE

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FN-ASSEMBLE-REFERRAL-CONTEXT	Function: Assemble Referral Context (p. 351)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-ASSEMBLE-REFERRAL-CONTEXT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-CONTEXT-QUERY	Service: Referral Context Query (p. 352)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-CONTEXT-QUERY
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FN-MANAGE-REFERRAL-PROGRESSION	Function: Manage Referral Progression (p. 352)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.FN-MANAGE-REFERRAL-PROGRESSION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-STATUS-OUTCOME-SERVICE	Service: Referral Status & Outcome Service (p. 353)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.SV-REFERRAL-STATUS-OUTCOME-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FN-RECEIVE-APPOINTMENT-NOTIFICATION	Function: Receive Appointment Notification (p. 360)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.FN-RECEIVE-APPOINTMENT-NOTIFICATION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-NOTIFICATION-INGRESS	Service: Appointment Notification Ingress (p. 360)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-NOTIFICATION-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.FN-SYNCHRONISE-APPOINTMENT-STATUS	Function: Synchronise Appointment Status (p. 361)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.FN-SYNCHRONISE-APPOINTMENT-STATUS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-SCHEDULE-QUERY	Service: Appointment Schedule Query (p. 362)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-SCHEDULING-ADMINISTRATION.SV-APPOINTMENT-SCHEDULE-QUERY
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-ESTABLISH-ENCOUNTER-CONTEXT	Function: Establish Encounter Context (p. 377)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-ESTABLISH-ENCOUNTER-CONTEXT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-CONTEXT-RESOLUTION	Service: Encounter Context Resolution (p. 378)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-CONTEXT-RESOLUTION

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-PROGRESS-ENCOUNTER-STATE	Function: Progress Encounter State (p. 378)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-PROGRESS-ENCOUNTER-STATE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LIFECYCLE-EVENT-NOTIFICATION	Service: Encounter Lifecycle Event Notification (p. 379)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LIFECYCLE-EVENT-NOTIFICATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-MAINTAIN-ENCOUNTER-CARE-PLACE-ASSOCIATION	Function: Maintain Encounter Care-Place Association (p. 380)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FN-MAINTAIN-ENCOUNTER-CARE-PLACE-ASSOCIATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LOCATION-HISTORY-QUERY	Service: Encounter Location History Query (p. 381)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.SV-ENCOUNTER-LOCATION-HISTORY-QUERY

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-RECEIVE-ORDER-REQUEST	Function: Receive Order Request (p. 391)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-RECEIVE-ORDER-REQUEST
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-REQUISITION-INGRESS	Service: Order Requisition Ingress (p. 391)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-REQUISITION-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-RESOLVE-ORDER-DESTINATION	Function: Resolve Order Destination (p. 392)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-RESOLVE-ORDER-DESTINATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-DISPATCH-SERVICE	Service: Order Dispatch Service (p. 393)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-DISPATCH-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-MANAGE-ORDER-PROGRESSION	Function: Manage Order Progression (p. 393)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-MANAGE-ORDER-PROGRESSION

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-STATUS-TRACKING-QUERY	Service: Order Status & Tracking Query (p. 394)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-STATUS-TRACKING-QUERY
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-COORDINATE-ORDER-MODIFICATION-CANCELLATION	Function: Coordinate Order Modification / Cancellation (p. 395)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-COORDINATE-ORDER-MODIFICATION-CANCELLATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-CANCELLATION-SERVICE	Service: Order Cancellation Service (p. 396)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-CANCELLATION-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FN-ASSOCIATE-ORDER-OUTCOME	Function: Associate Order Outcome (p. 396)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.FN-ASSOCIATE-ORDER-OUTCOME
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.SV-ORDER-OUTCOME-NOTIFICATION	Service: Order Outcome Notification (p. 397)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.SV-ORDER-OUTCOME-NOTIFICATION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FN-CORRELATE-DIAGNOSTIC-REQUEST	Function: Correlate Diagnostic Request (p. 413)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-CORRELATE-DIAGNOSTIC-REQUEST
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-CORRELATION-RESOLUTION	Service: Diagnostic Correlation Resolution (p. 414)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-CORRELATION-RESOLUTION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FN-BIND-DIAGNOSTIC-REPORT	Function: Bind Diagnostic Report (p. 414)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-BIND-DIAGNOSTIC-REPORT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-REPORT-INGRESS	Service: Diagnostic Report Ingress (p. 415)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-REPORT-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FN-DISTRIBUTE-DIAGNOSTIC-REPORT	Function: Distribute Diagnostic Report (p. 416)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.FN-DISTRIBUTE-DIAGNOSTIC-REPORT

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CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-RESULT-DELIVERY-SERVICE	Service: Diagnostic Result Delivery Service (p. 417)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.SV-DIAGNOSTIC-RESULT-DELIVERY-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ORDER	Function: Receive Medication Order (p. 431)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ORDER
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ORDER-INGRESS	Service: Medication Order Ingress (p. 431)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ORDER-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FN-TRACK-DISPENSE-EVENT	Function: Track Dispense Event (p. 432)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-TRACK-DISPENSE-EVENT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.SV-DISPENSE-EVENT-INGRESS	Service: Dispense Event Ingress (p. 433)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-DISPENSE-EVENT-INGRESS

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ADMINISTRATION-EVENT	Function: Receive Medication Administration Event (p. 434)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.FN-RECEIVE-MEDICATION-ADMINISTRATION-EVENT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ADMINISTRATION-INGRESS	Service: Medication Administration Ingress (p. 434)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.SV-MEDICATION-ADMINISTRATION-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURE-BOOKING-REQUEST	Function: Receive Procedure Booking / Request (p. 440)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURE-BOOKING-REQUEST
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURE-BOOKING-INGRESS	Service: Procedure Booking Ingress (p. 441)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURE-BOOKING-INGRESS

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURAL-DOCUMENTATION	Function: Receive Procedural Documentation (p. 442)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.FN-RECEIVE-PROCEDURAL-DOCUMENTATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURAL-DOCUMENTATION-INGRESS	Service: Procedural Documentation Ingress (p. 443)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-PROCEDURE-ADMINISTRATION.SV-PROCEDURAL-DOCUMENTATION-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-RECEIVE-CLINICAL-DOCUMENT	Function: Receive Clinical Document (p. 450)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-RECEIVE-CLINICAL-DOCUMENT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-INGRESS	Service: Clinical Document Ingress (p. 450)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-INGRESS

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-GOVERN-CLINICAL-DOCUMENT-LIFECYCLE	Function: Govern Clinical Document Lifecycle (p. 451)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-GOVERN-CLINICAL-DOCUMENT-LIFECYCLE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-LIFECYCLE-SERVICE	Service: Clinical Document Lifecycle Service (p. 452)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-CLINICAL-DOCUMENT-LIFECYCLE-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-INDEX-CLINICAL-DOCUMENT-METADATA	Function: Index Clinical Document Metadata (p. 452)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.FN-INDEX-CLINICAL-DOCUMENT-METADATA
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-DOCUMENT-METADATA-REGISTRY-QUERY	Service: Document Metadata Registry Query (p. 453)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.SV-DOCUMENT-METADATA-REGISTRY-QUERY

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FN-RECEIVE-CARE-PLAN	Function: Receive Care Plan (p. 472)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-RECEIVE-CARE-PLAN
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-INGRESS	Service: Care Plan Ingress (p. 472)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-INGRESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FN-ASSOCIATE-CARE-PLAN-CONTEXT	Function: Associate Care Plan Context (p. 473)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-ASSOCIATE-CARE-PLAN-CONTEXT
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CONTEXT-QUERY	Service: Care Plan Context Query (p. 474)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CONTEXT-QUERY
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FN-DISTRIBUTE-CARE-PLAN-CHANGE	Function: Distribute Care Plan Change (p. 474)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.FN-DISTRIBUTE-CARE-PLAN-CHANGE

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CHANGE-NOTIFICATION	Service: Care Plan Change Notification (p. 475)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.SV-CARE-PLAN-CHANGE-NOTIFICATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-DISTRIBUTE-CLINICAL-COMMUNICATION	Function: Distribute Clinical Communication (p. 482)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-DISTRIBUTE-CLINICAL-COMMUNICATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-SECURE-CLINICAL-MESSAGE-DISPATCH	Service: Secure Clinical Message Dispatch (p. 483)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-SECURE-CLINICAL-MESSAGE-DISPATCH
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-TRACK-BUSINESS-DELIVERY-ACKNOWLEDGEMENT	Function: Track Business Delivery Acknowledgement (p. 484)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-TRACK-BUSINESS-DELIVERY-ACKNOWLEDGEMENT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-DELIVERY-STATUS-QUERY	Service: Communication Delivery Status Query (p. 485)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-DELIVERY-STATUS-QUERY
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-RECORD-COMMUNICATION-TRANSACTION	Function: Record Communication Transaction (p. 485)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.FN-RECORD-COMMUNICATION-TRANSACTION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-AUDIT-QUERY	Service: Communication Audit Query (p. 486)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COMMUNICATION-ADMINISTRATION.SV-COMMUNICATION-AUDIT-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FN-CORRELATE-PRIMARY-CARE-CONSULTATION-CONTEXT	Function: Correlate Primary Care Consultation Context (p. 672)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FN-CORRELATE-PRIMARY-CARE-CONSULTATION-CONTEXT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.SV-PRIMARY-CARE-SUMMARY-INGRESS	Service: Primary Care Summary Ingress (p. 673)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.SV-PRIMARY-CARE-SUMMARY-INGRESS
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.FN-COORDINATE-MULTI-AGENCY-CHRONIC-CARE-PLAN	Function: Coordinate Multi-Agency Chronic Care Plan (p. 674)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.FN-COORDINATE-MULTI-AGENCY-CHRONIC-CARE-PLAN
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PRIMARY-CARE.SV-CHRONIC-CARE-TIMELINE-QUERY	Service: Chronic Care Timeline Query (p. 674)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-PRIMARY-CARE.SV-CHRONIC-CARE-TIMELINE-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FN-ASSEMBLE-ACUTE-CLINICAL-BASELINE	Function: Assemble Acute Clinical Baseline (p. 368)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FN-ASSEMBLE-ACUTE-CLINICAL-BASELINE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-ADMISSION-DOSSIER-QUERY	Service: Acute Admission Dossier Query (p. 369)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-ADMISSION-DOSSIER-QUERY

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.FN-ROUTE-ACUTE-CRITICAL-RESULT-ALERT	Function: Route Acute Critical Result Alert (p. 370)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.FN-ROUTE-ACUTE-CRITICAL-RESULT-ALERT
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-CLINICAL-ALERT-DISPATCH	Service: Acute Clinical Alert Dispatch (p. 370)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ACUTE-CARE.SV-ACUTE-CLINICAL-ALERT-DISPATCH
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FN-BIND-ED-TRIAGE-ASSESSMENT	Function: Bind ED Triage Assessment (p. 557)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FN-BIND-ED-TRIAGE-ASSESSMENT
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.SV-ED-TRIAGE-STREAM-INGRESS	Service: ED Triage Stream Ingress (p. 558)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.SV-ED-TRIAGE-STREAM-INGRESS
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.FN-PROVIDE-EMERGENCY-LHR-SUMMARY	Function: Provide Emergency LHR Summary (p. 558)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.FN-PROVIDE-EMERGENCY-LHR-SUMMARY

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-EMERGENCY-CARE.SV-EMERGENCY-FAST-PATH-LHR-QUERY	Service: Emergency Fast-Path LHR Query (p. 559)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-CARE.SV-EMERGENCY-FAST-PATH-LHR-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FN-CONSOLIDATE-INPATIENT-CLINICAL-ROUND-FEED	Function: Consolidate Inpatient Clinical Round Feed (p. 533)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FN-CONSOLIDATE-INPATIENT-CLINICAL-ROUND-FEED
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.SV-WARD-ROUND-CLINICAL-BRIEFING-QUERY	Service: Ward Round Clinical Briefing Query (p. 534)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.SV-WARD-ROUND-CLINICAL-BRIEFING-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.FN-ASSEMBLE-INPATIENT-DISCHARGE-DOSSIER	Function: Assemble Inpatient Discharge Dossier (p. 534)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.FN-ASSEMBLE-INPATIENT-DISCHARGE-DOSSIER
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-FACILITY-EPISODIC-CARE-INTEGRATION.CT3-BEC-INPATIENT-CARE.SV-DISCHARGE-DOSSIER-PACKAGING-SERVICE	Service: Discharge Dossier Packaging Service (p. 535)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-INPATIENT-CARE.SV-DISCHARGE-DOSSIER-PACKAGING-SERVICE

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-RECEIVE-DIAGNOSTIC-REQUISITION-FEED	Function: Receive Diagnostic Requisition Feed (p. 404)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-RECEIVE-DIAGNOSTIC-REQUISITION-FEED
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-MODALITY-WORKLIST-DISTRIBUTION	Service: Modality Worklist Distribution (p. 405)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-MODALITY-WORKLIST-DISTRIBUTION
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-PUBLISH-GOVERNED-DIAGNOSTIC-REPORT	Function: Publish Governed Diagnostic Report (p. 406)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.FN-PUBLISH-GOVERNED-DIAGNOSTIC-REPORT
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-DIAGNOSTIC-PUBLICATION-SERVICE	Service: Diagnostic Publication Service (p. 407)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-SERVICES.SV-DIAGNOSTIC-PUBLICATION-SERVICE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FN-PROJECT-LONGITUDINAL-MEDICATION-HISTORY	Function: Project Longitudinal Medication History (p. 423)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FN-PROJECT-LONGITUDINAL-MEDICATION-HISTORY

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.SV-LONGITUDINAL-MEDICATION-HISTORY-QUERY	Service: Longitudinal Medication History Query (p. 424)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.SV-LONGITUDINAL-MEDICATION-HISTORY-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.FN-CORRELATE-ADVERSE-REACTION-ALLERGY-PROFILE	Function: Correlate Adverse Reaction & Allergy Profile (p. 424)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.FN-CORRELATE-ADVERSE-REACTION-ALLERGY-PROFILE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-SPECIALIST-CLINICAL-CONTEXT-INTEGRATION.CT3-BEC-MEDICATION-THERAPY.SV-ALLERGY-ADVERSE-REACTION-QUERY	Service: Allergy & Adverse Reaction Query (p. 425)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-MEDICATION-ADMINISTRATION.CT3-BEC-MEDICATION-THERAPY.SV-ALLERGY-ADVERSE-REACTION-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FN-SYNDICATE-IMMUNISATION-RECORD	Function: Syndicate Immunisation Record (p. 333)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FN-SYNDICATE-IMMUNISATION-RECORD
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.SV-IMMUNISATION-HISTORY-QUERY	Service: Immunisation History Query (p. 334)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.SV-IMMUNISATION-HISTORY-QUERY

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.FN-PROJECT-SCREENING-ELIGIBILITY-TIMELINE	Function: Project Screening Eligibility Timeline (p. 334)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.FN-PROJECT-SCREENING-ELIGIBILITY-TIMELINE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-PREVENTIVE-CARE.SV-PREVENTIVE-SCREENING-STATUS-QUERY	Service: Preventive Screening Status Query (p. 335)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLIENT-CENTRED-ADMINISTRATION.CT3-BEC-PREVENTIVE-CARE.SV-PREVENTIVE-SCREENING-STATUS-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FN-SYNCHRONIZE-COMMUNITY-VISIT-RECORD	Function: Synchronize Community Visit Record (p. 463)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FN-SYNCHRONIZE-COMMUNITY-VISIT-RECORD
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-ENCOUNTER-INGRESS	Service: Community Care Encounter Ingress (p. 464)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-ENCOUNTER-INGRESS
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.FN-BIND-COMMUNITY-SUPPORT-NETWORK-CONTEXT	Function: Bind Community Support Network Context (p. 465)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.FN-BIND-COMMUNITY-SUPPORT-NETWORK-CONTEXT

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CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-TEAM-QUERY	Service: Community Care Team Query (p. 466)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-COMMUNITY-CARE.SV-COMMUNITY-CARE-TEAM-QUERY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FN-PROVIDE-OUTREACH-VISIT-CONTEXT	Function: Provide Outreach Visit Context (p. 470)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FN-PROVIDE-OUTREACH-VISIT-CONTEXT
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.FN-RECONCILE-OUTREACH-CLINICAL-BATCH	Function: Reconcile Outreach Clinical Batch (p. 470)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.FN-RECONCILE-OUTREACH-CLINICAL-BATCH
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-OUTREACH-CARE.SV-OUTREACH-BATCH-RECONCILIATION-SERVICE	Service: Outreach Batch Reconciliation Service (p. 471)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-OUTREACH-CARE.SV-OUTREACH-BATCH-RECONCILIATION-SERVICE
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-CORRELATE-VIRTUAL-CARE-CONSULTATION-CONTEXT	Function: Correlate Virtual Care Consultation Context (p. 679)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-CORRELATE-VIRTUAL-CARE-CONSULTATION-CONTEXT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-VIRTUAL-ENCOUNTER-CONTEXT-RESOLUTION	Service: Virtual Encounter Context Resolution (p. 679)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-VIRTUAL-ENCOUNTER-CONTEXT-RESOLUTION
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-INGEST-REMOTE-PATIENT-TELEMETRY	Function: Ingest Remote Patient Telemetry (p. 680)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.FN-INGEST-REMOTE-PATIENT-TELEMETRY
CT1-BEC-CARE-CONTEXT-INTEGRATION-ENABLEMENT.CT2-BEC-LONGITUDINAL-COMMUNITY-CARE-INTEGRATION.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-REMOTE-TELEMETRY-INGRESS	Service: Remote Telemetry Ingress (p. 681)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-REMOTE-VIRTUAL-CARE.SV-REMOTE-TELEMETRY-INGRESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-PRACTICE-CAPACITY	Function: Manage Clinic & Practice Capacity (p. 520)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-PRACTICE-CAPACITY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FN-TRACK-CLINIC-OPERATING-SESSION	Function: Track Clinic Operating Session (p. 520)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-TRACK-CLINIC-OPERATING-SESSION

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-QUEUE-PROGRESSION	Function: Manage Clinic Queue Progression (p. 521)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.FN-MANAGE-CLINIC-QUEUE-PROGRESSION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.SV-CLINIC-QUEUE-TELEMETRY-SERVICE	Service: Clinic Queue Telemetry Service (p. 522)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.SV-CLINIC-QUEUE-TELEMETRY-SERVICE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FN-MANAGE-WARD-CAPACITY	Function: Manage Ward Capacity (p. 536)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.FN-MANAGE-WARD-CAPACITY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.SV-WARD-OPERATIONAL-SUMMARY-QUERY	Service: Ward Operational Summary Query (p. 537)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.SV-WARD-OPERATIONAL-SUMMARY-QUERY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FN-ASSEMBLE-WARD-HANDOVER-CONTEXT	Function: Assemble Ward Handover Context (p. 537)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.FN-ASSEMBLE-WARD-HANDOVER-CONTEXT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CAPACITY	Function: Manage Theatre Capacity (p. 544)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CAPACITY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CASE-PROGRESSION	Function: Manage Theatre Case Progression (p. 545)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.FN-MANAGE-THEATRE-CASE-PROGRESSION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.SV-THEATRE-CASE-PROGRESSION-TELEMETRY	Service: Theatre Case Progression Telemetry (p. 546)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.SV-THEATRE-CASE-PROGRESSION-TELEMETRY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FN-COORDINATE-ED-DEPARTMENTAL-CAPACITY	Function: Coordinate ED Departmental Capacity (p. 560)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FN-COORDINATE-ED-DEPARTMENTAL-CAPACITY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.SV-ED-OPERATIONAL-STATUS-QUERY	Service: ED Operational Status Query (p. 561)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.SV-ED-OPERATIONAL-STATUS-QUERY

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FN-MANAGE-OUTPATIENT-CAPACITY	Function: Manage Outpatient Capacity (p. 567)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.FN-MANAGE-OUTPATIENT-CAPACITY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.SV-AMBULATORY-SESSION-STATUS-QUERY	Service: Ambulatory Session Status Query (p. 568)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.SV-AMBULATORY-SESSION-STATUS-QUERY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FN-CAPTURE-POST-CLINIC-FOLLOW-UP-REQUIREMENTS	Function: Capture Post-Clinic Follow-up Requirements (p. 569)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.FN-CAPTURE-POST-CLINIC-FOLLOW-UP-REQUIREMENTS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MAINTAIN-OPERATIONAL-BED-STATE	Function: Maintain Operational Bed State (p. 576)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MAINTAIN-OPERATIONAL-BED-STATE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-AVAILABILITY-QUERY	Service: Bed Availability Query (p. 577)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-AVAILABILITY-QUERY

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MANAGE-BED-TURNOVER-PROGRESSION	Function: Manage Bed Turnover Progression (p. 578)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.FN-MANAGE-BED-TURNOVER-PROGRESSION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-TURNOVER-LIFECYCLE-SERVICE	Service: Bed Turnover Lifecycle Service (p. 579)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.SV-BED-TURNOVER-LIFECYCLE-SERVICE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT.FN-TRACK-MOBILE-CLINICAL-ASSET-OPERATIONAL-STATE	Function: Track Mobile Clinical Asset Operational State (p. 314)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FN-TRACK-MOBILE-CLINICAL-ASSET-OPERATIONAL-STATE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-CLINICAL-RESOURCE-MANAGEMENT.SV-CLINICAL-RESOURCE-AVAILABILITY-QUERY	Service: Clinical Resource Availability Query (p. 315)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.SV-CLINICAL-RESOURCE-AVAILABILITY-QUERY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT.FN-AGGREGATE-SERVICE-CAPACITY-TELEMETRY	Function: Aggregate Service Capacity Telemetry (p. 585)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.FN-AGGREGATE-SERVICE-CAPACITY-TELEMETRY

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-SERVICE-CAPACITY-MANAGEMENT.SV-REGIONAL-HEALTH-SYSTEM-CAPACITY-QUERY	Service: Regional Health System Capacity Query (p. 585)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-SERVICE-CAPACITY-MANAGEMENT.SV-REGIONAL-HEALTH-SYSTEM-CAPACITY-QUERY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-MANAGE-MOBILE-SERVICE-CAPACITY	Function: Manage Mobile Service Capacity (p. 597)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-MANAGE-MOBILE-SERVICE-CAPACITY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-TRACK-OPERATIONAL-STAFF-PRESENCE	Function: Track Operational Staff Presence (p. 598)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-TRACK-OPERATIONAL-STAFF-PRESENCE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.SV-STAFF-PRESENCE-TELEMETRY-QUERY	Service: Staff Presence Telemetry Query (p. 599)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.SV-STAFF-PRESENCE-TELEMETRY-QUERY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-RECEIVE-MOBILE-VISIT-PROGRESSION	Function: Receive Mobile Visit Progression (p. 600)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-RECEIVE-MOBILE-VISIT-PROGRESSION

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FN-INTEGRATE-MOBILE-WORK-MANAGEMENT	Function: Integrate Mobile Work Management (p. 601)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.FN-INTEGRATE-MOBILE-WORK-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT.FN-RESOLVE-ACTIVE-ON-CALL-COVERAGE	Function: Resolve Active On-Call Coverage (p. 605)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.FN-RESOLVE-ACTIVE-ON-CALL-COVERAGE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-ON-CALL-MANAGEMENT.SV-ON-CALL-SPECIALIST-LOOKUP	Service: On-Call Specialist Lookup (p. 606)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-ON-CALL-MANAGEMENT.SV-ON-CALL-SPECIALIST-LOOKUP
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FN-DISPATCH-OPERATIONAL-WORK-ORDER	Function: Dispatch Operational Work Order (p. 614)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.FN-DISPATCH-OPERATIONAL-WORK-ORDER
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.SV-WORK-DISPATCH-SERVICE	Service: Work Dispatch Service (p. 614)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.SV-WORK-DISPATCH-SERVICE

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.FN-MANAGE-OPERATIONAL-WORK-PROGRESSION	Function: Manage Operational Work Progression (p. 615)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.FN-MANAGE-OPERATIONAL-WORK-PROGRESSION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.SV-OPERATIONAL-WORK-STATUS-QUERY	Service: Operational Work Status Query (p. 616)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.SV-OPERATIONAL-WORK-STATUS-QUERY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.FN-MANAGE-PATIENT-TRANSPORT-PROGRESSION	Function: Manage Patient Transport Progression (p. 637)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.FN-MANAGE-PATIENT-TRANSPORT-PROGRESSION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.SV-PATIENT-TRANSPORT-TRACKING-SERVICE	Service: Patient Transport Tracking Service (p. 638)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.SV-PATIENT-TRANSPORT-TRACKING-SERVICE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.FN-MANAGE-SPECIMEN-TRANSPORT-PROGRESSION	Function: Manage Specimen Transport Progression (p. 902)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.FN-MANAGE-SPECIMEN-TRANSPORT-PROGRESSION

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CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.SV-SPECIMEN-TRANSIT-TRACKING-SERVICE	Service: Specimen Transit Tracking Service (p. 644)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.SV-SPECIMEN-TRANSIT-TRACKING-SERVICE
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.FN-MANAGE-DISCHARGE-COORDINATION-PROGRESSION	Function: Manage Discharge Coordination Progression (p. 651)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.FN-MANAGE-DISCHARGE-COORDINATION-PROGRESSION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.SV-DISCHARGE-READINESS-TELEMETRY	Service: Discharge Readiness Telemetry (p. 652)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.SV-DISCHARGE-READINESS-TELEMETRY
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORKFORCE-QUALIFICATION-DEVELOPMENT-ENABLEMENT.CT3-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FN-VERIFY-CLINICAL-QUALIFICATION-FOR-WORK-DISPATCH	Function: Verify Clinical Qualification for Work Dispatch (p. 627)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-QUALIFICATION-MANAGEMENT.FN-VERIFY-CLINICAL-QUALIFICATION-FOR-WORK-DISPATCH

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FN-ASSEMBLE-LONGITUDINAL-CLINICAL-RECORD	Function: Assemble Longitudinal Clinical Record (p. 681)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-ASSEMBLE-LONGITUDINAL-CLINICAL-RECORD
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.SV-LONGITUDINAL-CLINICAL-RECORD-QUERY	Service: Longitudinal Clinical Record Query (p. 682)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-LONGITUDINAL-CLINICAL-RECORD-QUERY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FN-MAINTAIN-ACTIVE-CLINICAL-RECORD	Function: Maintain Active Clinical Record (p. 683)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-MAINTAIN-ACTIVE-CLINICAL-RECORD
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.SV-ACTIVE-PROBLEM-ALLERGY-SUMMARY-QUERY	Service: Active Problem & Allergy Summary Query (p. 684)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-ACTIVE-PROBLEM-ALLERGY-SUMMARY-QUERY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FN-PRESERVE-DURABLE-CLINICAL-RECORD	Function: Preserve Durable Clinical Record (p. 684)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.FN-PRESERVE-DURABLE-CLINICAL-RECORD

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.SV-HISTORICAL-CLINICAL-RECORD-ACCESS	Service: Historical Clinical Record Access (p. 685)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.SV-HISTORICAL-CLINICAL-RECORD-ACCESS
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-RECEIVE-INFORMATION-SUBMISSION	Function: Receive Information Submission (p. 695)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-RECEIVE-INFORMATION-SUBMISSION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-HIE-SUBMISSION-GATEWAY-SERVICE	Service: HIE Submission Gateway Service (p. 696)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-HIE-SUBMISSION-GATEWAY-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-MEDIATE-INFORMATION-RETRIEVAL	Function: Mediate Information Retrieval (p. 697)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-MEDIATE-INFORMATION-RETRIEVAL

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-FEDERATED-HEALTH-INFORMATION-QUERY	Service: Federated Health Information Query (p. 698)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-FEDERATED-HEALTH-INFORMATION-QUERY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-DISTRIBUTE-ADDRESSED-INFORMATION	Function: Distribute Addressed Information (p. 698)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-DISTRIBUTE-ADDRESSED-INFORMATION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-ADDRESSED-DOCUMENT-DELIVERY-SERVICE	Service: Addressed Document Delivery Service (p. 699)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-ADDRESSED-DOCUMENT-DELIVERY-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-SYNDICATE-INFORMATION-CHANGE	Function: Syndicate Information Change (p. 700)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.FN-SYNDICATE-INFORMATION-CHANGE

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-CLINICAL-EVENT-SYNDICATION-STREAM	Service: Clinical Event Syndication Stream (p. 700)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-EXCHANGE-ENABLEMENT.SV-CLINICAL-EVENT-SYNDICATION-STREAM
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FN-SEARCH-HEALTH-INFORMATION	Function: Search Health Information (p. 706)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-SEARCH-HEALTH-INFORMATION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-INFORMATION-SEARCH-SERVICE	Service: Clinical Information Search Service (p. 707)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-INFORMATION-SEARCH-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FN-RETRIEVE-HEALTH-INFORMATION	Function: Retrieve Health Information (p. 708)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-RETRIEVE-HEALTH-INFORMATION

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-RESOURCE-RETRIEVAL-SERVICE	Service: Clinical Resource Retrieval Service (p. 709)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.SV-CLINICAL-RESOURCE-RETRIEVAL-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONNECTIVITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-ACCESS.FN-APPLY-ACCESS-QUALIFIED-RESULT-FILTERING	Function: Apply Access-Qualified Result Filtering (p. 709)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-ACCESS.FN-APPLY-ACCESS-QUALIFIED-RESULT-FILTERING
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-INFORMATION-ACCESS-AUTHORITY	Function: Evaluate Information Access Authority (p. 722)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-INFORMATION-ACCESS-AUTHORITY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-POLICY-EVALUATION-AUTHORISATION-SERVICE	Service: Policy Evaluation & Authorisation Service (p. 723)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-POLICY-EVALUATION-AUTHORISATION-SERVICE

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-CONSENT-CONSTRAINT	Function: Evaluate Consent Constraint (p. 724)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-EVALUATE-CONSENT-CONSTRAINT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-CONSENT-ENFORCEMENT-SERVICE	Service: Consent Enforcement Service (p. 724)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-CONSENT-ENFORCEMENT-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-PROPAGATE-SECURITY-CONTEXT	Function: Propagate Security Context (p. 725)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-PROPAGATE-SECURITY-CONTEXT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-CONTEXT-VALIDATION-SERVICE	Service: Security Context Validation Service (p. 726)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-CONTEXT-VALIDATION-SERVICE

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.FN-RECORD-SECURITY-SIGNIFICANT-ACTIVITY	Function: Record Security-Significant Activity (p. 727)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.FN-RECORD-SECURITY-SIGNIFICANT-ACTIVITY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-AUDIT-INGRESS	Service: Security Audit Ingress (p. 727)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-HEALTH-INFORMATION-CONTROL.SV-SECURITY-AUDIT-INGRESS
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-RESOLVE-CLINICAL-CONCEPT	Function: Resolve Clinical Concept (p. 753)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-RESOLVE-CLINICAL-CONCEPT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-CONCEPT-RESOLUTION-SERVICE	Service: Terminology Concept Resolution Service (p. 754)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-CONCEPT-RESOLUTION-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-MAP-CLINICAL-CONCEPT	Function: Map Clinical Concept (p. 755)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.FN-MAP-CLINICAL-CONCEPT

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-MAPPING-SERVICE	Service: Terminology Mapping Service (p. 755)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.SV-TERMINOLOGY-MAPPING-SERVICE
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FN-COORDINATE-CARE-TEAM-COLLABORATION	Function: Coordinate Care-Team Collaboration (p. 763)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-COORDINATE-CARE-TEAM-COLLABORATION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.SV-CLINICAL-COLLABORATION-SPACE-SERVICE	Service: Clinical Collaboration Space Service (p. 764)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-CLINICAL-COLLABORATION-SPACE-SERVICE
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FN-RESOLVE-LHR-QUERY-WITHIN-COLLABORATION	Function: Resolve LHR Query within Collaboration (p. 765)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-RESOLVE-LHR-QUERY-WITHIN-COLLABORATION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-CLINICAL-SUMMARY-RESOLUTION	Service: Collaboration Clinical Summary Resolution (p. 766)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-CLINICAL-SUMMARY-RESOLUTION

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CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FN-GOVERN-COLLABORATION-METADATA	Function: Govern Collaboration Metadata (p. 766)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.FN-GOVERN-COLLABORATION-METADATA
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-SPACE-GOVERNANCE-SERVICE	Service: Collaboration Space Governance Service (p. 767)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.SV-COLLABORATION-SPACE-GOVERNANCE-SERVICE
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION.FN-COORDINATE-OPERATIONAL-COLLABORATION	Function: Coordinate Operational Collaboration (p. 772)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.FN-COORDINATE-OPERATIONAL-COLLABORATION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-GENERAL-COLLABORATION.SV-OPERATIONAL-COLLABORATION-SERVICE	Service: Operational Collaboration Service (p. 772)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-GENERAL-COLLABORATION.SV-OPERATIONAL-COLLABORATION-SERVICE
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FN-PROJECT-OPERATIONAL-SCHEDULE	Function: Project Operational Schedule (p. 783)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.FN-PROJECT-OPERATIONAL-SCHEDULE

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CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.SV-UNIFIED-CALENDAR-PROJECTION-QUERY	Service: Unified Calendar Projection Query (p. 784)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.SV-UNIFIED-CALENDAR-PROJECTION-QUERY
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.FN-CORRELATE-ACTIVITY-WITH-TEMPORAL-CONTEXT	Function: Correlate Activity with Temporal Context (p. 785)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.FN-CORRELATE-ACTIVITY-WITH-TEMPORAL-CONTEXT
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-OPERATIONAL-ACTIVITY-TEMPORAL-ENABLEMENT.CT3-BEC-CALENDAR-MANAGEMENT.SV-TEMPORAL-CONTEXT-RESOLUTION	Service: Temporal Context Resolution (p. 785)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CALENDAR-MANAGEMENT.SV-TEMPORAL-CONTEXT-RESOLUTION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES.FN-PRESENT-CONTEXTUAL-HEALTH-AND-OPERATIONAL-INFORMATION	Function: Present Contextual Health and Operational Information (p. 790)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.FN-PRESENT-CONTEXTUAL-HEALTH-AND-OPERATIONAL-INFORMATION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-PRESENTATION-SERVICES.SV-CONTEXTUAL-PRESENTATION-SERVICE	Service: Contextual Presentation Service (p. 791)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PRESENTATION-SERVICES.SV-CONTEXTUAL-PRESENTATION-SERVICE

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-SEMANTICS	Function: Govern Information Semantics (p. 803)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-SEMANTICS
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-MODEL-DEFINITION-SERVICE	Service: Semantic Model Definition Service (p. 804)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-MODEL-DEFINITION-SERVICE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-DEFINITION	Function: Govern Information Definition (p. 804)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-INFORMATION-DEFINITION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-INFORMATION-DICTIONARY-QUERY	Service: Information Dictionary Query (p. 805)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-INFORMATION-DICTIONARY-QUERY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-TERMINOLOGY-BINDING	Function: Govern Terminology Binding (p. 806)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-TERMINOLOGY-BINDING

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-TERMINOLOGY-BINDING-SPECIFICATION-LOOKUP	Service: Terminology Binding Specification Lookup (p. 806)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-TERMINOLOGY-BINDING-SPECIFICATION-LOOKUP
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-EXCHANGE-PROFILE	Function: Govern Exchange Profile (p. 807)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-GOVERN-EXCHANGE-PROFILE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-EXCHANGE-PROFILE-SPECIFICATION-QUERY	Service: Exchange Profile Specification Query (p. 808)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-EXCHANGE-PROFILE-SPECIFICATION-QUERY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-VERIFY-SEMANTIC-CONFORMANCE	Function: Verify Semantic Conformance (p. 808)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.FN-VERIFY-SEMANTIC-CONFORMANCE

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CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-CONFORMANCE-VERIFICATION-SERVICE	Service: Semantic Conformance Verification Service (p. 809)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.SV-SEMANTIC-CONFORMANCE-VERIFICATION-SERVICE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.PR-REFERRAL-PROGRESSION-PROCESS	Process: Referral Progression Process (p. 354)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.PR-REFERRAL-PROGRESSION-PROCESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.PR-ENCOUNTER-LIFECYCLE-PROCESS	Process: Encounter Lifecycle Process (p. 381)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.PR-ENCOUNTER-LIFECYCLE-PROCESS
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.PR-CLOSED-LOOP-ORDER-PROGRESSION-PROCESS	Process: Closed-Loop Order Progression Process (p. 398)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.PR-CLOSED-LOOP-ORDER-PROGRESSION-PROCESS

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CLINICAL-RECORD-ADMINISTRATION.PR-CLINICAL-DOCUMENT-LIFECYCLE-PROCESS	Process: Clinical Document Lifecycle Process (p. 454)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-RECORD-ADMINISTRATION.PR-CLINICAL-DOCUMENT-LIFECYCLE-PROCESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.PR-THEATRE-CASE-PROGRESSION-PROCESS	Process: Theatre Case Progression Process (p. 547)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.PR-THEATRE-CASE-PROGRESSION-PROCESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.PR-CLINIC-OPERATIONAL-QUEUE-PROGRESSION-PROCESS	Process: Clinic / Operational Queue Progression Process (p. 523)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.PR-CLINIC-OPERATIONAL-QUEUE-PROGRESSION-PROCESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-RESOURCE-CAPACITY-COORDINATION.CT3-BEC-BED-CARE-PLACE-MANAGEMENT.PR-BED-TURNOVER-PROCESS	Process: Bed Turnover Process (p. 580)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-BED-CARE-PLACE-MANAGEMENT.PR-BED-TURNOVER-PROCESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-WORK-ALLOCATION-DISPATCH.PR-OPERATIONAL-WORK-PROGRESSION-PROCESS	Process: Operational Work Progression Process (p. 617)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WORK-ALLOCATION-DISPATCH.PR-OPERATIONAL-WORK-PROGRESSION-PROCESS

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-PATIENT-TRANSPORT.PR-PATIENT-TRANSPORT-PROCESS	Process: Patient Transport Process (p. 638)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-PATIENT-TRANSPORT.PR-PATIENT-TRANSPORT-PROCESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-CLINICAL-LOGISTICS-COORDINATION.PR-SPECIMEN-TRANSPORT-PROCESS	Process: Specimen Transport Process (p. 916)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINICAL-LOGISTICS-COORDINATION.PR-SPECIMEN-TRANSPORT-PROCESS
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-MOVEMENT-TRANSITION-COORDINATION.CT3-BEC-DISCHARGE-MANAGEMENT.PR-DISCHARGE-PROGRESSION-PROCESS	Process: Discharge Progression Process (p. 652)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-DISCHARGE-MANAGEMENT.PR-DISCHARGE-PROGRESSION-PROCESS
EC-01	Enterprise CT2: Managed Entity & Relationship (p. 838)	CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-01
EC-02	Enterprise CT2: Context Management (p. 839)	CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-02
EC-03	Enterprise CT2: Managed State & Lifecycle (p. 842)	CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-03
EC-04	Enterprise CT2: Information Management (p. 844)	CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-04
EC-05	Enterprise CT2: Search & Discovery (p. 849)	CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-05
EC-06	Enterprise CT2: Policy & Control (p. 865)	CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-06
EC-07	Enterprise CT2: Provenance & Traceability (p. 869)	CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-07
EC-08	Enterprise CT2: Interoperability & Exchange (p. 854)	CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-08
EC-09	Enterprise CT2: Event & Subscription (p. 856)	CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-09
EC-10	Enterprise CT2: Activity & Execution (p. 857)	CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-10
EC-11	Enterprise CT2: Interaction & Experience (p. 862)	CT1-EC-COORDINATED-ACTIVITY-INTERACTION.CT2-EC-11
EC-12	Enterprise CT2: Operational Assurance (p. 873)	CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-12

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EC-13	Enterprise CT2: Semantic Governance & Conformance (p. 851)	CT1-EC-GOVERNED-INFORMATION-OPERATIONS.CT2-EC-13
EC-14	Enterprise CT2: Service Guardian (p. 875)	CT1-EC-TRUST-ACCOUNTABILITY-ASSURANCE.CT2-EC-14
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT	BEC CT1: Health Informatics Management (p. 811)	CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT	BEC CT3: Linked Information Curation (p. 663)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION	BEC CT3: Governed Reference Dataset Administration (p. 748)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP	BEC CT3: Data Provenance Capture (p. 733)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION	BEC CT2: Research Information Curation (p. 812)	CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT	BEC CT2: Population Health Information Reporting (p. 817)	CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-SERVICE-PLANNING-INFORMATION-PROVISION	BEC CT2: Service Planning Data Management (p. 818)	CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-SERVICE-PLANNING-INFORMATION-PROVISION
CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-ASSOCIATION.FEAT-STEP2-DEVICE-ASSOCIATION-LINEAGE	Feature: Device Association Lineage (p. 315)	CT1-BEC-HEALTHCARE-ENTITY-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-DEVICE-ADMINISTRATION.CT3-BEC-DEVICE-UTILISATION-ADMINISTRATION.FEAT-STEP2-DEVICE-ASSOCIATION-LINEAGE
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-ACCESS-TRANSACTION-ENABLEMENT.CT3-BEC-REFERRAL-ADMINISTRATION.FEAT-STEP2-REFERRAL-DESTINATION-COORDINATION	Feature: Referral Destination Coordination (p. 348)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-REFERRAL-ADMINISTRATION.CT3-BEC-REFERRAL-INTAKE-PROGRESSION.FEAT-STEP2-REFERRAL-DESTINATION-COORDINATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.FEAT-STEP2-ENCOUNTER-HISTORY-RETRIEVAL	Feature: Encounter History Retrieval (p. 376)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-EPISODE-ENCOUNTER-ADMINISTRATION.CT3-BEC-ENCOUNTER-LIFECYCLE-MANAGEMENT.FEAT-STEP2-ENCOUNTER-HISTORY-RETRIEVAL

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-ORDER-ADMINISTRATION.FEAT-STEP2-ORDER-OUTCOME-ASSOCIATION	Feature: Order Outcome Association (p. 390)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-ORDER-ADMINISTRATION.CT3-BEC-CLOSED-LOOP-ORDER-PROGRESSION.FEAT-STEP2-ORDER-OUTCOME-ASSOCIATION
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-FULFILMENT-ADMINISTRATION-ENABLEMENT.CT3-BEC-DIAGNOSTIC-ADMINISTRATION.FEAT-STEP2-DIAGNOSTIC-PRELIMINARY-CORRECTED-REPORT-HANDLING	Feature: Diagnostic Preliminary & Corrected Report Handling (p. 412)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-DIAGNOSTIC-ADMINISTRATION.CT3-BEC-DIAGNOSTIC-WORKFLOW-MANAGEMENT.FEAT-STEP2-DIAGNOSTIC-PRELIMINARY-CORRECTED-REPORT-HANDLING
CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-DOCUMENTATION-COMMUNICATION-ENABLEMENT.CT3-BEC-CARE-PLAN-ADMINISTRATION.FEAT-STEP2-CARE-GOAL-PLAN-PROGRESSION-REPRESENTATION	Feature: Care Goal & Plan Progression Representation (p. 459)	CT1-BEC-CARE-ADMINISTRATION-INFORMATION-ENABLEMENT.CT2-BEC-CARE-PLAN-ADMINISTRATION.CT3-BEC-CARE-PLAN-LIFECYCLE-COORDINATION.FEAT-STEP2-CARE-GOAL-PLAN-PROGRESSION-REPRESENTATION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-PATIENT-CLINICAL-RECORD.FEAT-STEP2-CLINICAL-INFORMATION-HISTORICAL-RECONSTRUCTION	Feature: Clinical Information Historical Reconstruction (p. 662)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-LONGITUDINAL-CLINICAL-RECORD-ADMINISTRATION.FEAT-STEP2-CLINICAL-INFORMATION-HISTORICAL-RECONSTRUCTION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FEAT-STEP2-BINARY-CONTENT-RESOLUTION-ASSEMBLY	Feature: Binary Content Resolution & Assembly (p. 665)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FEAT-STEP2-BINARY-CONTENT-RESOLUTION-ASSEMBLY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-STEP2-GOVERNED-DATASET-CONTENT-REVISION	Feature: Governed Dataset Content Revision (p. 750)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FEAT-STEP2-GOVERNED-DATASET-CONTENT-REVISION
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FEAT-STEP2-INFORMATION-TRANSFORMATION-LINEAGE	Feature: Information Transformation Lineage (p. 735)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FEAT-STEP2-INFORMATION-TRANSFORMATION-LINEAGE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-STEP2-INFORMATION-DEFINITION-SCHEMA-GOVERNANCE	Feature: Information Definition & Schema Governance (p. 796)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-INFORMATION-DEFINITION-SCHEMA-GOVERNANCE

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-STEP2-TERMINOLOGY-BINDING-GOVERNANCE	Feature: Terminology Binding Governance (p. 797)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-TERMINOLOGY-BINDING-GOVERNANCE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-INFORMATION-DESIGN-GOVERNANCE.FEAT-STEP2-EXCHANGE-PROFILE-GOVERNANCE	Feature: Exchange Profile Governance (p. 798)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-DESIGN-GOVERNANCE.CT3-BEC-ENTERPRISE-CANONICAL-REUSABLE-MODEL-DESIGN.FEAT-STEP2-EXCHANGE-PROFILE-GOVERNANCE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FEAT-STEP2-AUTHORISED-RESEARCH-RELEASE-ASSEMBLY	Feature: Authorised Research Release Assembly (p. 739)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-AUTHORISED-RESEARCH-RELEASE-ASSEMBLY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-POPULATION-HEALTH-NOTIFICATION-ENABLEMENT.FEAT-STEP2-POPULATION-NOTIFICATION-OBLIGATION-TRACKING	Feature: Population Notification Obligation Tracking (p. 740)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-POPULATION-NOTIFICATION-OBLIGATION-TRACKING

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-SERVICE-PLANNING-INFORMATION-PROVISION.FEAT-STEP2-DEMAND-CAPACITY-PLANNING-PROJECTION	Feature: Demand & Capacity Planning Projection (p. 741)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-PRODUCT-ADMINISTRATION.FEAT-STEP2-DEMAND-CAPACITY-PLANNING-PROJECTION
CT1-BEC-HUMAN-COLLABORATION-WORK-ENABLEMENT.CT2-BEC-COLLABORATIVE-INTERACTION-ENABLEMENT.CT3-BEC-CLINICAL-COLLABORATION.FEAT-STEP2-COLLABORATION-INTERACTION-CAPTURE-QUALIFICATION	Feature: Collaboration Interaction Capture Qualification (p. 762)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-COLLABORATION.CT3-BEC-CLINICAL-CONTEXT-COLLABORATION.FEAT-STEP2-COLLABORATION-INTERACTION-CAPTURE-QUALIFICATION
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-CLINIC-PRACTICE-OPERATIONS.FEAT-STEP2-CLINIC-SESSION-CAPACITY-MANAGEMENT	Feature: Clinic Session Capacity Management (p. 519)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CLINIC-PRACTICE-OPERATIONS.CT3-BEC-SESSION-QUEUE-COORDINATION.FEAT-STEP2-CLINIC-SESSION-CAPACITY-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-WARD-OPERATIONS.FEAT-STEP2-WARD-CARE-CAPACITY-MANAGEMENT	Feature: Ward Care Capacity Management (p. 528)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-WARD-OPERATIONS.CT3-BEC-WARD-LOGISTICS-COORDINATION.FEAT-STEP2-WARD-CARE-CAPACITY-MANAGEMENT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-THEATRE-OPERATIONS.FEAT-STEP2-THEATRE-SESSION-CAPACITY-MANAGEMENT	Feature: Theatre Session Capacity Management (p. 543)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-THEATRE-OPERATIONS.CT3-BEC-PERIOPERATIVE-LOGISTICS-COORDINATION.FEAT-STEP2-THEATRE-SESSION-CAPACITY-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.FEAT-STEP2-EMERGENCY-TREATMENT-CAPACITY-MANAGEMENT	Feature: Emergency Treatment Capacity Management (p. 553)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-EMERGENCY-DEPARTMENT-OPERATIONS.CT3-BEC-EMERGENCY-DEPARTMENT-OPERATIONAL-COORDINATION.FEAT-STEP2-EMERGENCY-TREATMENT-CAPACITY-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-CARE-SETTING-OPERATIONS-ENABLEMENT.CT3-BEC-OUTPATIENT-OPERATIONS.FEAT-STEP2-OUTPATIENT-APPOINTMENT-CAPACITY-MANAGEMENT	Feature: Outpatient Appointment Capacity Management (p. 566)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OUTPATIENT-OPERATIONS.CT3-BEC-OUTPATIENT-FLOW-COORDINATION.FEAT-STEP2-OUTPATIENT-APPOINTMENT-CAPACITY-MANAGEMENT
CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-OPERATIONAL-WORKFORCE-COORDINATION.CT3-BEC-MOBILE-STAFF-MANAGEMENT.FEAT-STEP2-MOBILE-SERVICE-CAPACITY-MANAGEMENT	Feature: Mobile Service Capacity Management (p. 596)	CT1-BEC-CARE-OPERATIONS-COORDINATION-ENABLEMENT.CT2-BEC-MOBILE-STAFF-MANAGEMENT.CT3-BEC-MOBILE-WORKER-DISPATCH-SAFETY-COORDINATION.FEAT-STEP2-MOBILE-SERVICE-CAPACITY-MANAGEMENT

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-RESOLVE-ASSOCIATED-BINARY-CONTENT	Function: Resolve Associated Binary Content (p. 666)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-RESOLVE-ASSOCIATED-BINARY-CONTENT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-ASSEMBLE-ASSOCIATED-BINARY-CONTENT	Function: Assemble Associated Binary Content (p. 667)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.FN-ASSEMBLE-ASSOCIATED-BINARY-CONTENT
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-INFORMATION-STEWARDSHIP-ENABLEMENT.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.SV-ASSOCIATED-BINARY-CONTENT-RETRIEVAL	Service: Associated Binary Content Retrieval (p. 668)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-PATIENT-CLINICAL-RECORD.CT3-BEC-ASSOCIATED-CLINICAL-BINARY-CONTENT-ENABLEMENT.SV-ASSOCIATED-BINARY-CONTENT-RETRIEVAL
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FN-ACCEPT-GOVERNED-DATASET-REVISION	Function: Accept Governed Dataset Revision (p. 751)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.FN-ACCEPT-GOVERNED-DATASET-REVISION

Historical ID alias	Current identity	Current structural ID
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SEMANTIC-REFERENCE-KNOWLEDGE-ENABLEMENT.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.SV-GOVERNED-REFERENCE-DATASET-CONTENT-QUERY	Service: Governed Reference Dataset Content Query (p. 752)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-CLINICAL-KNOWLEDGE-SERVICES.CT3-BEC-GOVERNED-REFERENCE-DATASET-ADMINISTRATION.SV-GOVERNED-REFERENCE-DATASET-CONTENT-QUERY
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-PROTECTION-ACCOUNTABILITY-ENABLEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FN-RECORD-SIGNIFICANT-INFORMATION-LINEAGE	Function: Record Significant Information Lineage (p. 736)	CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-INFORMATION-CONTENT-MAINTAINER-MANAGEMENT.CT3-BEC-INFORMATION-LINEAGE-STEWARDSHIP.FN-RECORD-SIGNIFICANT-INFORMATION-LINEAGE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FN-ASSEMBLE-PURPOSE-QUALIFIED-RESEARCH-RELEASE	Function: Assemble Purpose-Qualified Research Release (p. 814)	CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.FN-ASSEMBLE-PURPOSE-QUALIFIED-RESEARCH-RELEASE
CT1-BEC-GOVERNED-HEALTH-INFORMATION-ENABLEMENT.CT2-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT3-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.SV-AUTHORISED-RESEARCH-INFORMATION-EXPORT	Service: Authorised Research Information Export (p. 815)	CT1-BEC-SECONDARY-HEALTH-INFORMATION-USE-ENABLEMENT.CT2-BEC-AUTHORISED-RESEARCH-INFORMATION-PROVISION.SV-AUTHORISED-RESEARCH-INFORMATION-EXPORT

35. v0.3 concept reconciliation

All 942 primary concepts and 277 captured behaviour occurrences were inspected against the v0.1 substrate and current responsibility boundaries. The machine-readable reconciliation retains an individual disposition, exact source line, target identities and rationale for each. Old working IDs do not become aliases, and generated pattern repetition does not create Canonical responsibility.

Disposition	Concept count
Retained Canonical	340
Retained Proposed	5
Reclassified scope	1
Retained Candidate	1
Reconciled to Canonical	1
Not retained as a separate responsibility	2
Reclassified participation	1
Reclassified concern	2
Reclassified enablement	10
Reconciled concern	1
Reclassified change concern	1
Rejected implementation-shaped grouping	1
Retained external/roadmap concern	1
Reclassified integration concern	1
Reconciled to human-reviewed hierarchy	5
Retained Canonical responsibility	80
Subsumed concern; no distinct Feature delta	316
Refined / repositioned	15
Retained Proposed responsibility	45
Reconciled predecessor — consolidated	9
Reconciled predecessor — decomposed	2
Removed non-element placeholder	4
Reconciled predecessor — superseded by reviewed capability	1
HUMAN ADJUDICATION REQUIRED	4
Integrated reusable responsibility	25
Reclassified recurrence evidence	68

35.1. Business

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-E85E18F4` Individual Care Delivery (line 65)	Retained Canonical	BC CT1: Individual Care Delivery
`BC-AA0D9B50` Individual Health Assessment (line 82)	Retained Canonical	BC CT2: Individual Health Assessment
`BC-6956B3FA` Health History Assessment (line 94)	Retained Canonical	BC CT3: Health History Assessment
`BC-69CE36B4` Clinical Examination (line 105)	Retained Canonical	BC CT3: Clinical Examination
`BC-3F88F485` Health Status Measurement (line 116)	Retained Canonical	BC CT3: Health Status Measurement

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BC-201A989E Clinical Determination (line 127)	Retained Canonical	BC CT2: Clinical Determination
BC-20CA91E6 Clinical Finding Interpretation (line 139)	Retained Proposed	BC CT3: Clinical Finding Interpretation
BC-A1AC4A2A Diagnosis & Problem Determination (line 150)	Retained Proposed	BC CT3: Diagnosis & Problem Determination
BC-75E01B0D Health Risk & Prognostic Determination (line 161)	Retained Proposed	BC CT3: Health Risk & Prognostic Determination
BC-F657B57A Individual Care Planning (line 172)	Retained Canonical	BC CT2: Individual Care Planning
BC-E8BBA49D Care Goal Definition (line 183)	Retained Proposed	BC CT3: Care Goal Definition
BC-3626AD34 Care Response Planning (line 194)	Retained Proposed	BC CT3: Care Response Planning
BC-3CBC77C1 Clinical Intervention (line 205)	Retained Canonical	BC CT2: Clinical Intervention
BC-DD466A23 Individual Health Monitoring (line 217)	Retained Canonical	BC CT2: Individual Health Monitoring
BC-7E2744D3 Health State Monitoring (line 228)	Retained Canonical	BC CT3: Health State Monitoring
BC-02C5DBC7 Care Response Monitoring (line 239)	Retained Canonical	BC CT3: Care Response Monitoring
BC-391EC3B9 Individual Care Review (line 250)	Retained Canonical	BC CT2: Individual Care Review
BC-A52E0A33 Care Appropriateness Review (line 261)	Retained Canonical	BC CT3: Care Appropriateness Review
BC-31953E77 Care Effectiveness Review (line 272)	Retained Canonical	BC CT3: Care Effectiveness Review
BC-2C769A48 Care Access & Coordination (line 283)	Retained Canonical	BC CT1: Care Access & Coordination
BC-FA41A886 Care Access Management (line 298)	Retained Canonical	BC CT2: Care Access Management
BC-267F1D1B Care Intake Management (line 310)	Retained Canonical	BC CT3: Care Intake Management
BC-EA6B7EB9 Care Referral Management (line 321)	Retained Canonical	BC CT3: Care Referral Management
BC-4F461126 Care Eligibility Determination (line 332)	Retained Canonical	BC CT3: Care Eligibility Determination
BC-FE20C7C9 Care Scheduling & Capacity Management (line 343)	Retained Canonical	BC CT2: Care Scheduling & Capacity Management
BC-9000E197 Care Appointment Management (line 358)	Retained Canonical	BC CT3: Care Appointment Management
BC-A1F26127 Care-Place Allocation (line 369)	Retained Canonical	BC CT3: Care-Place Allocation
BC-3284993B Care-Place Readiness Management (line 380)	Retained Canonical	BC CT3: Care-Place Readiness Management
BC-B53CD721 Care Resource Deployment (line 391)	Retained Canonical	BC CT3: Care Resource Deployment

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BC-E24E4ABE Operational Care Flow Coordination (line 402)	Retained Canonical	BC CT3: Operational Care Flow Coordination
BC-C2D4C33E Operational Care Capacity Management (line 413)	Retained Canonical	BC CT3: Operational Care Capacity Management
BC-59CFFB65 Care Coordination & Transition (line 424)	Retained Canonical	BC CT2: Care Coordination & Transition
BC-9C9BD5F2 Care-Team Coordination (line 436)	Retained Canonical	BC CT3: Care-Team Coordination
BC-AA12612C Care Responsibility Transition (line 447)	Retained Canonical	BC CT3: Care Responsibility Transition
BC-D9B48397 Care Discharge Coordination (line 458)	Retained Canonical	BC CT3: Care Discharge Coordination
BC-C313E0F4 Care Support Logistics Management (line 469)	Retained Canonical	BC CT2: Care Support Logistics Management
BC-CE1271E6 Patient Movement Coordination (line 481)	Retained Canonical	BC CT3: Patient Movement Coordination
BC-8924ED80 Clinical Material Logistics Coordination (line 492)	Retained Canonical	BC CT3: Clinical Material Logistics Coordination
BC-E0FD8D9D Care Support Work Coordination (line 503)	Retained Canonical	BC CT3: Care Support Work Coordination
BC-3E1A6DCA Health Rights, Advocacy & Participation (line 514)	Retained Canonical	BC CT1: Health Rights, Advocacy & Participation
BC-C705C8AD Diagnostic, Therapeutic & Clinical Support Services (line 526)	Retained Canonical	BC CT1: Diagnostic, Therapeutic & Clinical Support Services
BC-E0BE8A8A Medication & Pharmacy Services (line 539)	Retained Canonical	BC CT2: Medication & Pharmacy Services
BC-4DA49AEE Prescription Medication Dispensing (line 552)	Retained Canonical	BC CT3: Prescription Medication Dispensing
BC-C67D1E73 Non-Prescription Medication Provision (line 563)	Retained Canonical	BC CT3: Non-Prescription Medication Provision
BC-4E4274D4 Medication Advice (line 574)	Retained Canonical	BC CT3: Medication Advice
BC-F8E65CD9 Medication Dose Administration Aid Provision (line 585)	Retained Canonical	BC CT3: Medication Dose Administration Aid Provision
BC-7B292540 Diagnostic Support Provision (line 598)	Retained Canonical	BC CT2: Diagnostic Support Provision
BC-D7778EF0 Diagnostic Service Appointment Management (line 614)	Retained Canonical	BC CT3: Diagnostic Service Appointment Management
BC-20131F3B Diagnostic Service Delivery (line 625)	Retained Canonical	BC CT3: Diagnostic Service Delivery

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-D29EA624` Diagnostic Specimen Management (line 636)	Retained Canonical	BC CT3: Diagnostic Specimen Management
`BC-47BE7761` Diagnostic Data Management (line 647)	Retained Canonical	BC CT3: Diagnostic Data Management
`BC-30AB08BB` Diagnostic Interpretation (line 658)	Retained Canonical	BC CT3: Diagnostic Interpretation
`BC-E3C0477C` Diagnostic Report Management (line 669)	Retained Canonical	BC CT3: Diagnostic Report Management
`BC-E001550A` Diagnostic Report Distribution (line 680)	Retained Canonical	BC CT3: Diagnostic Report Distribution
`BC-C82D3CC6` Health Products & Clinical Technology (line 691)	Retained Canonical	BC CT1: Health Products & Clinical Technology
`BC-D7FFDEDC` Health Product Management (line 704)	Reclassified scope	BEC CT2: Health Materiel Administration
`BC-377FEB37` Clinical Technology Management (line 714)	Retained Canonical	BC CT2: Clinical Technology Management
`BC-097846F2` Clinical Device Inventory Management (line 731)	Retained Canonical	BC CT3: Clinical Device Inventory Management
`BC-E511B2DB` Clinical Device Feature & Release Management (line 742)	Retained Canonical	BC CT3: Clinical Device Feature & Release Management
`BC-E2E1C257` Clinical Device Loan & Deployment Management (line 753)	Retained Canonical	BC CT3: Clinical Device Loan & Deployment Management
`BC-BBE81736` Clinical Device Utilisation Support (line 764)	Retained Canonical	BC CT3: Clinical Device Utilisation Support
`BC-33CA15F2` Clinical Device Integration Provision (line 775)	Retained Canonical	BC CT3: Clinical Device Integration Provision
`BC-4F2A8587` Clinical Device Data Integration (line 786)	Retained Canonical	BC CT3: Clinical Device Data Integration
`BC-6CB8AC63` Clinical Device Maintenance Management (line 797)	Retained Candidate	BC CT3: Clinical Device Maintenance Management
`BC-1BFF3F5B` Clinical Device Lifecycle Management (line 808)	Retained Canonical	BC CT3: Clinical Device Lifecycle Management
`BC-E95C383F` Clinical Quality, Safety & Improvement (line 821)	Retained Canonical	BC CT1: Clinical Quality, Safety & Improvement
`BC-5B2A0E55` Clinical Safety Management (line 834)	Retained Canonical	BC CT2: Clinical Safety Management
`BC-7E3FBC64` Clinical Risk Identification (line 846)	Retained Canonical	BC CT3: Clinical Risk Identification
`BC-5121C6F1` Clinical Risk Mitigation Management (line 857)	Retained Canonical	BC CT3: Clinical Risk Mitigation Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-72DCF354` Clinical Risk Response Management (line 868)	Retained Canonical	BC CT3: Clinical Risk Response Management
`BC-077229CB` Clinical Quality Management (line 879)	Retained Canonical	BC CT2: Clinical Quality Management
`BC-807007A1` Clinical Quality Criteria Management (line 892)	Retained Canonical	BC CT3: Clinical Quality Criteria Management
`BC-FC14D2E2` Clinical Quality Evidence Management (line 903)	Retained Canonical	BC CT3: Clinical Quality Evidence Management
`BC-6884DABF` Clinical Quality Review & Assessment Management (line 914)	Retained Canonical	BC CT3: Clinical Quality Review & Assessment Management
`BC-6EEB86B4` Clinical Quality Reporting (line 925)	Retained Canonical	BC CT3: Clinical Quality Reporting
`BC-31C8DC2C` Community Health & Wellbeing (line 936)	Retained Canonical	BC CT1: Community Health & Wellbeing
`BC-80DA636C` Preventative Health (line 948)	Retained Canonical	BC CT2: Preventative Health
`BC-1FAD6263` Preventative Health Problem Identification (line 963)	Retained Canonical	BC CT3: Preventative Health Problem Identification
`BC-B3BDD9A1` Preventative Health Programme Management (line 974)	Retained Canonical	BC CT3: Preventative Health Programme Management
`BC-57B54EBE` Preventative Health Education Services (line 985)	Retained Canonical	BC CT3: Preventative Health Education Services
`BC-5202AEF0` Preventative Health Programme Monitoring (line 996)	Retained Canonical	BC CT3: Preventative Health Programme Monitoring
`BC-E55D2726` Preventative Health Reporting (line 1007)	Retained Canonical	BC CT3: Preventative Health Reporting
`BC-1800BCD9` Preventative Health Cohort Management (line 1018)	Retained Canonical	BC CT3: Preventative Health Cohort Management
`BC-28CCEE27` Population Health & Health-System Planning (line 1029)	Retained Canonical	BC CT1: Population Health & Health-System Planning
`BC-A6F4CA49` Population Health Surveillance (line 1042)	Retained Canonical	BC CT2: Population Health Surveillance
`BC-F1D5D26C` Population Health Longitudinal Assessment (line 1056)	Retained Canonical	BC CT3: Population Health Longitudinal Assessment
`BC-AFC31F01` Population Health Surveillance Management (line 1067)	Retained Canonical	BC CT3: Population Health Surveillance Management
`BC-BCA30D47` Population Health Event Identification (line 1078)	Retained Canonical	BC CT3: Population Health Event Identification
`BC-5EAA6955` Notifiable Health Event Management (line 1089)	Retained Canonical	BC CT3: Notifiable Health Event Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BC-D8C15299 Population Health Reporting (line 1100)	Retained Canonical	BC CT3: Population Health Reporting
BC-40B6A2C7 Health-System Planning (line 1111)	Retained Canonical	BC CT2: Health-System Planning
BC-770F60DC Health Service Demand Assessment (line 1123)	Retained Canonical	BC CT3: Health Service Demand Assessment
BC-3742724C Health Service Capacity Planning (line 1134)	Retained Canonical	BC CT3: Health Service Capacity Planning
BC-33A5DE96 Health Service Distribution Planning (line 1145)	Retained Canonical	BC CT3: Health Service Distribution Planning
BC-D37DBD07 Health Information & Knowledge Management (line 1156)	Retained Canonical	BC CT1: Health Information & Knowledge Management
BC-E21B0F21 Health Information Management (line 1169)	Retained Canonical	BC CT2: Health Information Management
BC-05A55E6B Health Information Data Management (line 1185)	Retained Canonical	BC CT3: Health Information Data Management
BC-822903B0 Health Information Structure Management (line 1196)	Retained Canonical	BC CT3: Health Information Structure Management
BC-65531F2E Health Information Lifecycle Management (line 1207)	Retained Canonical	BC CT3: Health Information Lifecycle Management
BC-2C668F82 Health Information Catalogue and Index Management (line 1218)	Retained Canonical	BC CT3: Health Information Catalogue and Index Management
BC-AD69E2A8 Health Information Compartmentalisation (line 1229)	Retained Canonical	BC CT3: Health Information Compartmentalisation
BC-39F5CC28 Health Information Data Correlation (line 1240)	Retained Canonical	BC CT3: Health Information Data Correlation
BC-C6E88499 Health Information Data Retrieval (line 1251)	Retained Canonical	BC CT3: Health Information Data Retrieval
BC-55C02BD4 Clinical Knowledge Management (line 1262)	Retained Canonical	BC CT2: Clinical Knowledge Management
BC-16799A80 Clinical Knowledge Context Identification (line 1277)	Retained Canonical	BC CT3: Clinical Knowledge Context Identification
BC-D45CD0AA Clinical Knowledge Context Extraction (line 1288)	Retained Canonical	BC CT3: Clinical Knowledge Context Extraction
BC-385FCDAE Clinical Knowledge Information Formulation (line 1299)	Retained Canonical	BC CT3: Clinical Knowledge Information Formulation
BC-AD456750 Clinical Knowledge Concept Association (line 1310)	Retained Canonical	BC CT3: Clinical Knowledge Concept Association
BC-88D82704 Clinical Knowledge Review & Approval Management (line 1321)	Retained Canonical	BC CT3: Clinical Knowledge Review & Approval Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BC-E57A9C37 Clinical Knowledge Publication & Sharing (line 1332)	Retained Canonical	BC CT3: Clinical Knowledge Publication & Sharing
BC-014363F4 Standards, Semantics & Reference Governance (line 1343)	Retained Canonical	BC CT1: Standards, Semantics & Reference Governance
BC-C01C01B9 Health Information Standards Governance (line 1356)	Retained Canonical	BC CT2: Health Information Standards Governance
BC-C9D7E7C2 Information Model Governance (line 1376)	Reconciled to Canonical	BC CT3: Information Model Governance Oversight
BC-F55F16A6 Information Model Discovery (line 1387)	Retained Canonical	BC CT3: Information Model Discovery
BC-B27B41ED Information Model Adoption (line 1398)	Retained Canonical	BC CT3: Information Model Adoption
BC-C5E3E979 Information Model Education (line 1409)	Retained Canonical	BC CT3: Information Model Education
BC-FF2BF6BE Information Model Compliance (line 1420)	Retained Canonical	BC CT3: Information Model Compliance
BC-4486449F Standards Discovery (line 1431)	Not retained as a separate responsibility	BC CT2: Health Information Standards Governance
BC-98A00C71 Standards Impact Identification (line 1442)	Retained Canonical	BC CT3: Standards Impact Identification
BC-CB858814 Standards Adoption (line 1453)	Retained Canonical	BC CT3: Standards Adoption
BC-CB56B8BC Standards Education (line 1464)	Not retained as a separate responsibility	BC CT2: Health Information Standards Governance
BC-BFC63CC1 Standards Bodies Engagement (line 1475)	Reclassified participation	BC CT2: Health Information Standards Governance
BC-E515BE41 Standards Compliance (line 1486)	Reclassified concern	BC CT2: Health Information Standards Governance
BC-F9F83F32 Clinical Semantics Governance (line 1497)	Retained Canonical	BC CT2: Clinical Semantics Governance
BC-9D2AC8A5 Terminology Governance (line 1512)	Retained Canonical	BC CT3: Terminology Governance
BC-9DBB18D4 Classification Governance (line 1523)	Retained Canonical	BC CT3: Classification Governance
BC-05135118 Semantic Mapping Governance (line 1534)	Retained Canonical	BC CT3: Semantic Mapping Governance
BC-18835F13 Terminology Utilisation Oversight (line 1545)	Retained Canonical	BC CT3: Terminology Utilisation Oversight
BC-D4198837 Terminology Localisation (line 1556)	Retained Canonical	BC CT3: Terminology Localisation
BC-A6FDE957 Classification Utilisation Oversight (line 1567)	Retained Canonical	BC CT3: Classification Utilisation Oversight

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-D0174B04` Connected Health Services (line 1578)	Retained Canonical	BC CT1: Connected Health Services
`BC-1A2C0473` Health Information Exchange (line 1592)	Retained Canonical	BC CT2: Health Information Exchange
`BC-25F9B730` Standards-Based Interoperability Exploration (line 1608)	Reclassified concern	BC CT2: Health Information Exchange
`BC-5CA9556E` Standards-Based Interoperability Support (line 1619)	Reclassified enablement	BEC CT2: Health Information Exchange Enablement
`BC-9D0C5F85` Standards-Based Interoperability Publication (line 1630)	Reclassified enablement	BEC CT3: Health Information Communication
`BC-EA4804CA` Standards-Based Interoperability Client-Support (line 1641)	Reclassified enablement	BEC CT2: Health Information Exchange Enablement
`BC-EAE75CE5` Health Information Sharing Framework Oversight (line 1652)	Reconciled concern	BC CT3: Health Information Sharing Programme Management
`BC-F6DF2559` Health Information Sharing Client-Support (line 1663)	Reclassified enablement	BEC CT2: Health Information Exchange Enablement
`BC-93BAE720` Health Information Sharing Release Management (line 1674)	Reclassified change concern	BC CT3: Health Information Sharing Programme Management
`BC-5FD7002B` Digital Communication Channel (line 1685)	Retained Canonical	BC CT2: Digital Communication Channel
`BC-1FA8828A` Clinical Message Conveyance (line 1697)	Reclassified enablement	BEC CT2: Clinical Communication Administration
`BC-BAEB9E26` Health Event Notification (line 1708)	Reclassified enablement	BEC CT2: Health Information Exchange Enablement; BEC CT2: Population Health Information Reporting
`BC-B4211FE9` Health Service Collaboration (line 1719)	Reclassified enablement	BEC CT2: Clinical Collaboration; BEC CT2: General Collaboration
`BC-1079B553` Domain Health Information Services (line 1730)	Rejected implementation-shaped grouping	BEC CT1: Entity Information Enablement; BEC CT1: Shared Information & Coordination Enablement
`BC-34251AB8` Provider Directory Services (line 1744)	Reclassified enablement	BEC CT2: Provider Administration; Feature: Provider Directory Discovery & Resolution
`BC-667FBAA5` Enterprise Master Patient Index Services (line 1755)	Retained external/roadmap concern	BEC CT3: Person Identity Administration
`BC-E3F3D922` Immunisation Register Services (line 1766)	Reclassified integration concern	BEC CT3: Screening and Immunisation Enablement
`BC-EDB9A9D1` Longitudinal Health Record Services (line 1777)	Reclassified enablement	BEC CT2: Patient Clinical Record
`BC-A60C3D92` Collaboration Services (line 1788)	Reclassified enablement	BEC CT2: Clinical Collaboration; BEC CT2: General Collaboration
`BC-3525B6FE` Security, Privacy & Digital Trust (line 1799)	Retained Canonical	BC CT1: Security, Privacy & Digital Trust

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-0D570829` Digital Identity & Access Governance (line 1813)	Retained Canonical	BC CT2: Digital Identity & Access Governance
`BC-51F3C14E` Identity Assurance (line 1830)	Retained Canonical	BC CT3: Identity Assurance
`BC-E2872CBE` Identity Modelling & Interoperability (line 1841)	Retained Canonical	BC CT3: Identity Modelling & Interoperability
`BC-D6F6BD86` Identity Qualification Control (line 1852)	Retained Canonical	BC CT3: Identity Qualification Control
`BC-2C37A4F9` Identity Identification & Authentication (line 1863)	Retained Canonical	BC CT3: Identity Identification & Authentication
`BC-92E77AA5` Access Control Context Management (line 1874)	Retained Canonical	BC CT3: Access Control Context Management
`BC-0E134F87` Access Control Assurance & Reporting (line 1885)	Retained Canonical	BC CT3: Access Control Assurance & Reporting
`BC-3A9DB312` Access Control Collaboration (line 1896)	Retained Canonical	BC CT3: Access Control Collaboration
`BC-A78294CD` Access Control Enforcement (line 1907)	Retained Canonical	BC CT3: Access Control Enforcement
`BC-448EE064` Health Information Protection (line 1918)	Retained Canonical	BC CT2: Health Information Protection
`BC-F8DE8E42` Health Privacy Safeguard Management (line 1932)	Retained Canonical	BC CT3: Health Privacy Safeguard Management
`BC-CBD883DA` Health Information Safeguarding (line 1943)	Retained Canonical	BC CT3: Health Information Safeguarding
`BC-88AD2C7F` Health Information Access Monitoring (line 1954)	Retained Canonical	BC CT3: Health Information Access Monitoring
`BC-69A112EB` Health Information Compartment Assurance (line 1965)	Retained Canonical	BC CT3: Health Information Compartment Assurance
`BC-C163719D` Health Information Classification Assurance (line 1976)	Retained Canonical	BC CT3: Health Information Classification Assurance
`BC-DB00A738` Digital Accountability & Trust (line 1987)	Retained Canonical	BC CT2: Digital Accountability & Trust
`BC-B2FBA48B` Governed Activity Attribution (line 1998)	Retained Canonical	BC CT3: Governed Activity Attribution
`BC-C26E8578` Security & Information-Use Accountability (line 2009)	Retained Canonical	BC CT3: Security & Information-Use Accountability
`BC-C5ED32F7` Health Research & Innovation (line 2020)	Retained Canonical	BC CT1: Health Research & Innovation
`BC-5FD1C13A` Research Information Support (line 2033)	Retained Canonical	BC CT2: Research Information Support

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BC-3C3F6346 Governed Research Information Provision (line 2043)	Retained Canonical	BC CT3: Governed Research Information Provision
BC-5B3EE7BD Health Innovation (line 2054)	Retained Canonical	BC CT2: Health Innovation
BC-E804A440 Health Innovation Evaluation (line 2064)	Retained Canonical	BC CT3: Health Innovation Evaluation
BC-A7B13259 Health Service Direction & Stewardship (line 2075)	Retained Canonical	BC CT1: Health Service Direction & Stewardship
BC-25BC222B Health Service Modelling (line 2088)	Retained Canonical	BC CT2: Health Service Modelling
BC-DF112315 Health Service Need Identification (line 2101)	Retained Canonical	BC CT3: Health Service Need Identification
BC-A7A90EE0 Health Service Capability Identification (line 2112)	Retained Canonical	BC CT3: Health Service Capability Identification
BC-EB94288C Health Service Implementation Support (line 2123)	Retained Canonical	BC CT3: Health Service Implementation Support
BC-B1242610 Health Service Continuous Improvement (line 2134)	Retained Canonical	BC CT3: Health Service Continuous Improvement
BC-8229ABFD Health Service Alignment (line 2145)	Retained Canonical	BC CT2: Health Service Alignment
BC-60910383 Health Service Co-Dependency Management (line 2158)	Retained Canonical	BC CT3: Health Service Co-Dependency Management
BC-AC53A525 Health Service Resource Identification (line 2169)	Retained Canonical	BC CT3: Health Service Resource Identification
BC-93FDB4F7 Health Service Financial Management (line 2180)	Retained Canonical	BC CT3: Health Service Financial Management
BC-EF0176F3 Health Service Implementation Planning (line 2191)	Retained Canonical	BC CT3: Health Service Implementation Planning
BC-F02F310B Workforce & Organisational Capability (line 2202)	Retained Canonical	BC CT1: Workforce & Organisational Capability
BC-B7DE9919 Health Workforce Management (line 2215)	Retained Canonical	BC CT2: Health Workforce Management
BC-875377CD Practitioner Qualification Management (line 2227)	Retained Canonical	BC CT3: Practitioner Qualification Management
BC-93AF8CF1 Practitioner Engagement & Privilege Management (line 2238)	Retained Canonical	BC CT3: Practitioner Engagement & Privilege Management
BC-DB75D205 Workforce Service Coverage Management (line 2249)	Retained Canonical	BC CT3: Workforce Service Coverage Management
BC-DC260B97 Healthcare Organisation & Service Structure Management (line 2260)	Retained Canonical	BC CT2: Healthcare Organisation & Service Structure Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-A170CFA2` Healthcare Organisation Identity & Structure Management (line 2272)	Retained Canonical	BC CT3: Healthcare Organisation Identity & Structure Management
`BC-65079689` Healthcare Location Definition Management (line 2283)	Retained Canonical	BC CT3: Healthcare Location Definition Management
`BC-40A6EEE3` Healthcare Service Offering Management (line 2294)	Retained Canonical	BC CT3: Healthcare Service Offering Management
`BC-2A4887A2` Corporate Resources & Enterprise Services (line 2305)	Retained Canonical	BC CT1: Corporate Resources & Enterprise Services
`BC-3618D6D1` Financial & Commercial Services (line 2318)	Retained Canonical	BC CT2: Financial & Commercial Services
`BC-924A98C9` Clinical Activity Charging Information Provision (line 2328)	Retained Canonical	BC CT3: Clinical Activity Charging Information Provision
`BC-F411AF90` Corporate Support Services (line 2339)	Retained Canonical	BC CT2: Corporate Support Services
`BC-FA7FBBAD` Back-Office Services (line 2354)	Retained Canonical	BC CT3: Back-Office Services
`BC-C8C4C54F` Facilities Management (line 2365)	Retained Canonical	BC CT3: Facilities Management
`BC-EDD1575C` IT Infrastructure Management (line 2376)	Retained Canonical	BC CT3: IT Infrastructure Management
`BC-08AE4743` Communications & Publicity (line 2387)	Retained Canonical	BC CT3: Communications & Publicity
`BC-4406E5B2` People Management (line 2398)	Retained Canonical	BC CT3: People Management
`BC-CF3A8B92` Operational Oversight (line 2409)	Retained Canonical	BC CT3: Operational Oversight
`BC-1EF62F79` Health Service Governance (line 2420)	Retained Canonical	BC CT1: Health Service Governance
`BC-5998319B` Governance Framework Management (line 2433)	Retained Canonical	BC CT2: Governance Framework Management
`BC-C01FF756` Policy / Obligation Governance (line 2444)	Retained Canonical	BC CT3: Policy / Obligation Governance
`BC-B3AE7E9A` Authority & Constraint Governance (line 2455)	Retained Canonical	BC CT3: Authority & Constraint Governance
`BC-15EBFFF4` Compliance Governance (line 2466)	Retained Canonical	BC CT2: Compliance Governance
`BC-9BEBD16E` Compliance Assessment Definition (line 2477)	Retained Canonical	BC CT3: Compliance Assessment Definition
`BC-351A6321` Governance Reporting & Checkpoint Definition (line 2488)	Retained Canonical	BC CT3: Governance Reporting & Checkpoint Definition
`BC-62520466` Health Service Assurance (line 2499)	Retained Canonical	BC CT1: Health Service Assurance

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BC-A0266334` Health Service Assurance Specification (line 2512)	Retained Canonical	BC CT2: Health Service Assurance Specification
`BC-BA00F585` Assurance Approach Definition (line 2523)	Retained Canonical	BC CT3: Assurance Approach Definition
`BC-28F181C3` Assurance Evaluation Criteria Governance (line 2534)	Retained Canonical	BC CT3: Assurance Evaluation Criteria Governance
`BC-11ACE749` Assurance Evaluation (line 2545)	Retained Canonical	BC CT2: Assurance Evaluation
`BC-EF506E30` Assurance Evidence Assessment (line 2556)	Retained Canonical	BC CT3: Assurance Evidence Assessment
`BC-BC9E1E00` Assurance Conclusion Determination (line 2567)	Retained Canonical	BC CT3: Assurance Conclusion Determination

35.2. Business Enabling

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-F49A0D09` Entity Information Enablement (line 2579)	Reconciled to human-reviewed hierarchy	BEC CT1: Entity Information Enablement
`BEC-75CEC238` Client Administration (line 2595)	Retained Canonical responsibility	BEC CT2: Client Administration
`BEC-31E69256` Person Identity (line 2608)	Retained Canonical responsibility	BEC CT3: Person Identity Administration
`BEC-1ACD822D` Identifier Resolution (line 2628)	Retained Canonical	Feature: Identifier Resolution
`BEC-3E1D6BA5` Cross-Authority Identifier Correlation (line 2641)	Retained Canonical	Feature: Cross-Authority Identifier Correlation
`BEC-905C884B` Identity Alias Association (line 2652)	Retained Canonical	Feature: Identity Alias Association
`BEC-F44A13C3` Governed Identity Correction (line 2662)	Retained Canonical	Feature: Governed Identity Correction
`BEC-60F846CB` Identity Management (line 2674)	Subsumed concern; no distinct Feature delta	BEC CT3: Person Identity Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-D5D59967` Identity Correlation (line 2686)	Refined / repositioned	Feature: Cross-Authority Identifier Correlation
`BEC-44123E71` Identity Correction (line 2698)	Refined / repositioned	Feature: Governed Identity Correction
`BEC-C7981054` Relationship Management (line 2710)	Subsumed concern; no distinct Feature delta	BEC CT3: Person Identity Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-07EB6044` Identity Discovery (line 2722)	Subsumed concern; no distinct Feature delta	BEC CT3: Person Identity Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-EDCE7C88 Healthcare Subject (line 2735)	Retained Canonical responsibility	BEC CT3: Healthcare Subject Profile Administration
BEC-19B283F0 Subject Context Binding (line 2750)	Retained Canonical	Feature: Subject Context Binding
BEC-6B8C0FDF Demographic Context Governance (line 2761)	Retained Canonical	Feature: Demographic Context Governance
BEC-98ED0FC6 Context Establishment (line 2772)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Subject Profile Administration; Enterprise CT2: Context Management
BEC-B3C6EFA0 State Monitoring (line 2782)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Subject Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-7C07A1C7 Exception Management (line 2792)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Subject Profile Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-54EAE8CA Information Retrieval (line 2802)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Subject Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-2494865F Client Relationship (line 2813)	Retained Canonical responsibility	BEC CT3: Client Relationship Administration
BEC-BD118344 Next-of-Kin & Guardian Association (line 2830)	Retained Canonical	Feature: Next-of-Kin & Guardian Association
BEC-6811FCDB Relationship Validity Tracking (line 2841)	Retained Canonical	Feature: Relationship Validity Tracking
BEC-CDB17807 Context Establishment (line 2852)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Relationship Administration; Enterprise CT2: Context Management
BEC-D61B3C3B State Monitoring (line 2862)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Relationship Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-4AACC1B3 Exception Management (line 2872)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Relationship Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-C9ABFA6F Information Retrieval (line 2882)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Relationship Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-ADC0CF33 Client Privacy (line 2893)	Retained Canonical responsibility	BEC CT3: Client Privacy Guardrail Management
BEC-F41B7E86 Consent Directive Evaluation (line 2911)	Retained Canonical	Feature: Consent Directive Evaluation
BEC-4D74B3D2 Confidentiality Flag Enforcement (line 2922)	Retained Canonical	Feature: Confidentiality Flag Enforcement
BEC-AA3166D7 Criteria Management (line 2933)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Privacy Guardrail Management; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-5A06BA3C Evidence Association (line 2943)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Privacy Guardrail Management; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-EF1AF8F9 Policy Evaluation (line 2953)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Privacy Guardrail Management; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-7D40E369 Decision Traceability (line 2963)	Refined / repositioned	Feature: Mandatory Significant Activity Audit Capture
BEC-FAF82B77 Exception Reporting (line 2973)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Privacy Guardrail Management; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-A4E759E2 Provider Administration (line 2984)	Retained Canonical responsibility	BEC CT2: Provider Administration
BEC-5BB55046 Practitioner (line 2997)	Retained Proposed responsibility	BEC CT3: Practitioner Profile Administration
BEC-95B0A32B National Practitioner Verification (line 3015)	Retained Canonical	Feature: National Practitioner Verification
BEC-7672710F Practitioner Profile Governance (line 3027)	Retained Canonical	Feature: Practitioner Profile Governance
BEC-A4CAE7D6 Identity Management (line 3038)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-C7BBD105 Identity Correlation (line 3050)	Refined / repositioned	Feature: Cross-Authority Identifier Correlation
BEC-F9FA105B Qualification Association (line 3062)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-028ABD17 Professional Relationship Management (line 3074)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-E9C766CB Practitioner Discovery (line 3086)	Refined / repositioned	Feature: Provider Directory Discovery & Resolution
BEC-2DEC073A Practitioner Role (line 3099)	Retained Proposed responsibility	BEC CT3: Practitioner Role Administration
BEC-116F8108 Role & Specialty Modeling (line 3117)	Retained Canonical	Feature: Role & Specialty Modeling
BEC-CBDE3B8C Electronic Endpoint Binding (line 3128)	Retained Canonical	Feature: Electronic Endpoint Binding
BEC-7F9BCB4F Identity Management (line 3139)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-314BE2D1 Identity Correlation (line 3151)	Refined / repositioned	Feature: Cross-Authority Identifier Correlation

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-A4596B27 Qualification Association (line 3163)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-549D792B Professional Relationship Management (line 3175)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-E24857D9 Practitioner Discovery (line 3187)	Refined / repositioned	Feature: Provider Directory Discovery & Resolution
BEC-C410CC51 Provider Relationship (line 3200)	Reconciled predecessor — consolidated	BEC CT3: Practitioner Role Administration
BEC-F5495DD1 Practitioner-Organisation Affiliation (line 3216)	Retained Canonical	Feature: Practitioner-Organisation Affiliation
BEC-0027885B Context Establishment (line 3227)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Context Management
BEC-2CF899D2 State Monitoring (line 3237)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-9806E183 Exception Management (line 3247)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-737E5A3F Information Retrieval (line 3257)	Subsumed concern; no distinct Feature delta	BEC CT3: Practitioner Role Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-F58DBA08 Provider Engagement (line 3268)	Reconciled predecessor — decomposed	BEC CT3: Practitioner Role Administration; BEC CT3: Provider Clinical Qualification Tracking
BEC-E0307698 Clinical Privileging Record (line 3284)	Retained Canonical	Feature: Clinical Privileging Record
BEC-0C362F3D Context Establishment (line 3295)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Context Management
BEC-141E9549 State Monitoring (line 3305)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-788A21BF Exception Management (line 3315)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-167AF187 Information Retrieval (line 3325)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-1FF88A71 Organisation Administration (line 3336)	Retained Canonical responsibility	BEC CT2: Organisation Administration
BEC-ACE86B9B Healthcare Organisation (line 3348)	Retained Proposed responsibility	BEC CT3: Healthcare Organisation Profile Administration

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-28102E35` National Organisation Verification (line 3366)	Retained Canonical	Feature: National Organisation Verification
`BEC-DF977EAB` Organisation Profile Management (line 3377)	Retained Canonical	Feature: Organisation Profile Management
`BEC-157386A8` Identity Management (line 3388)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-9262FB93` Identity Correlation (line 3400)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-AD09D42A` Organisational Relationship Management (line 3412)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-6B51A123` Organisational Structure Management (line 3424)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-D6FB2614` Organisation Discovery (line 3436)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-FA161A55` Organisational Structure (line 3449)	Reconciled predecessor — consolidated	BEC CT3: Healthcare Organisation Profile Administration
`BEC-F5068C04` Hierarchical Structure Navigation (line 3466)	Retained Canonical	Feature: Hierarchical Structure Navigation
`BEC-48E4C563` Identity Management (line 3477)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-A50F5059` Identity Correlation (line 3489)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-676858F6` Organisational Relationship Management (line 3501)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-ACAB1168` Organisational Structure Management (line 3513)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-721B38D1` Organisation Discovery (line 3525)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-5C12BF05` Organisation Contact (line 3538)	Reconciled predecessor — consolidated	BEC CT3: Healthcare Organisation Profile Administration

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-D6FA9FDF Directory Contact Management (line 3555)	Retained Canonical	Feature: Directory Contact Management
BEC-3FD95735 Identity Management (line 3566)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-737C2F3E Identity Correlation (line 3578)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-EFB7F137 Organisational Relationship Management (line 3590)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-272EFC75 Organisational Structure Management (line 3602)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-73EF81CD Organisation Discovery (line 3614)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Organisation Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-D4B0D057 Location Administration (line 3627)	Retained Canonical responsibility	BEC CT2: Location Administration
BEC-7E83F611 Healthcare Location (line 3639)	Retained Proposed responsibility	BEC CT3: Healthcare Location Profile Administration
BEC-885CE86B Location Geospatial & Address Resolution (line 3655)	Retained Canonical	Feature: Location Geospatial & Address Resolution
BEC-15887E90 Location Identity Management (line 3666)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-C52417EE Location Hierarchy Management (line 3678)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-0538C23A Location Availability Management (line 3690)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-BE1B64CD Location Discovery (line 3702)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-A66483F0 Location Structure (line 3715)	Reconciled predecessor — consolidated	BEC CT3: Healthcare Location Profile Administration
BEC-C82C8D0A Physical Hierarchy Modeling (line 3731)	Retained Canonical	Feature: Physical Hierarchy Modeling
BEC-FC2DD94F Location Identity Management (line 3742)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-6DCC28F3 Location Hierarchy Management (line 3754)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-310FF956 Location Availability Management (line 3766)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-4BCD6B9C Location Discovery (line 3778)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Location Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-0C8D2717 Care-Place Definition (line 3791)	Retained Proposed responsibility	BEC CT3: Care-Place Definition Administration
BEC-6322E089 Atomic Care-Place Modeling (line 3807)	Retained Canonical	Feature: Atomic Care-Place Modeling
BEC-705CD00C Context Establishment (line 3818)	Subsumed concern; no distinct Feature delta	BEC CT3: Care-Place Definition Administration; Enterprise CT2: Context Management
BEC-67DFF33A State Monitoring (line 3828)	Subsumed concern; no distinct Feature delta	BEC CT3: Care-Place Definition Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-E79AFE62 Exception Management (line 3838)	Subsumed concern; no distinct Feature delta	BEC CT3: Care-Place Definition Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-4B7196D5 Information Retrieval (line 3848)	Subsumed concern; no distinct Feature delta	BEC CT3: Care-Place Definition Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-B3668209 Health Service Administration (line 3859)	Retained Canonical responsibility	BEC CT2: Health Service Administration
BEC-1864D570 Healthcare Service Definition (line 3872)	Retained Proposed responsibility	BEC CT3: Healthcare Service Profile Administration
BEC-5AF74389 Service Catalogue Publishing (line 3888)	Retained Canonical	Feature: Service Catalogue Publishing
BEC-8CDB717F Service Definition Management (line 3899)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Semantic Governance & Conformance
BEC-766061BE Service Eligibility Management (line 3911)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-22CF71DF Service Availability Management (line 3923)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-DEE34CB8 Service Discovery (line 3935)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-CC9FD951 Service Provision Relationship (line 3948)	Reconciled predecessor — consolidated	BEC CT3: Healthcare Service Profile Administration
BEC-C68165B1 Service-Location-Provider Binding (line 3964)	Retained Canonical	Feature: Service-Location-Provider Binding
BEC-46B095FB Service Definition Management (line 3975)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Semantic Governance & Conformance
BEC-99951D40 Service Eligibility Management (line 3987)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-F964694F Service Availability Management (line 3999)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-7A743DDD Service Discovery (line 4011)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-CF667CC1 Service Availability Definition (line 4024)	Reconciled predecessor — consolidated	BEC CT3: Healthcare Service Profile Administration
BEC-A4ABC149 Operating Schedule Specification (line 4040)	Retained Canonical	Feature: Operating Schedule Specification
BEC-61F28669 Service Definition Management (line 4051)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Semantic Governance & Conformance
BEC-0AA34D18 Service Eligibility Management (line 4063)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-235C211D Service Availability Management (line 4075)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-B8A2142A Service Discovery (line 4087)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-8858C964 Service Eligibility Definition (line 4100)	Reconciled predecessor — consolidated	BEC CT3: Healthcare Service Profile Administration
BEC-748B065D Eligibility & Catchment Specification (line 4116)	Retained Canonical	Feature: Eligibility & Catchment Specification
BEC-271154E8 Service Definition Management (line 4127)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Semantic Governance & Conformance

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-1F711951 Service Eligibility Management (line 4139)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-FA68F289 Service Availability Management (line 4151)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-9B1E4AC5 Service Discovery (line 4163)	Subsumed concern; no distinct Feature delta	BEC CT3: Healthcare Service Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-60F2ADD5 Health Product Administration (line 4176)	Retained Canonical responsibility	BEC CT2: Health Materiel Administration
BEC-1BE2ADD6 Health Product Definition (line 4188)	Retained Proposed responsibility	BEC CT3: Health Materiel Profile Administration
BEC-3700FBFB No established Feature decomposition (Adjacent / Reference context) (line 4204)	Removed non-element placeholder	Non-element placeholder
BEC-0EECBF8C Context Establishment (line 4214)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Profile Administration; Enterprise CT2: Context Management
BEC-8F3D543E State Monitoring (line 4224)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-6F386B2A Exception Management (line 4234)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Profile Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-F792B00B Information Retrieval (line 4244)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Profile Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-F3F076F6 Product Formulary & Catalogue (line 4255)	Retained Proposed responsibility	BEC CT3: Health Materiel Catalogue & Formulary Administration
BEC-4937CA4F No established Feature decomposition (Adjacent / Reference context) (line 4270)	Removed non-element placeholder	Non-element placeholder
BEC-D5CD867C Identity Resolution (line 4280)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Catalogue & Formulary Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-A315E2F3 Identity Correlation (line 4290)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Catalogue & Formulary Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-3DBACC5A Relationship Management (line 4300)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Catalogue & Formulary Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-5C469882` Lifecycle Management (line 4310)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Catalogue & Formulary Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-9B063E58` Discovery (line 4320)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Catalogue & Formulary Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-0ACE1412` Product Batch & Serial Tracking (line 4331)	Retained Proposed responsibility	BEC CT3: Health Materiel Traceability Administration
`BEC-4614D02A` No established Feature decomposition (Adjacent / Reference context) (line 4345)	Removed non-element placeholder	Non-element placeholder
`BEC-A2361427` Context Establishment (line 4355)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Traceability Administration; Enterprise CT2: Context Management
`BEC-C2E441ED` State Monitoring (line 4365)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Traceability Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-DB97A930` Exception Management (line 4375)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Traceability Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-4C113522` Information Retrieval (line 4385)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Materiel Traceability Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-39C0D38D` Clinical Device Administration (line 4396)	Retained Canonical responsibility	BEC CT2: Clinical Device Administration
`BEC-EF278D25` Device Definition (line 4409)	Retained Proposed responsibility	BEC CT3: Clinical Device Profile Administration
`BEC-784E5062` Device Type Specification (line 4425)	Retained Canonical	Feature: Device Type Specification
`BEC-3117C7A3` Device Identity Management (line 4436)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-9F1FBF5C` Device State Management (line 4448)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-018C4C3F` Device Relationship Management (line 4460)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-624E75EE` Device Provenance Tracking (line 4472)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Provenance & Traceability
`BEC-B3DEDCD2` Device Instance (line 4485)	Reconciled predecessor — consolidated	BEC CT3: Clinical Device Profile Administration
`BEC-8FB4909A` Device Serial Tracking (line 4501)	Retained Canonical	Feature: Device Serial Tracking

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-A111F20F` Device Identity Management (line 4512)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-8C9D3C83` Device State Management (line 4524)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-A8E9A77A` Device Relationship Management (line 4536)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-EB9AE886` Device Provenance Tracking (line 4548)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Device Profile Administration; Enterprise CT2: Provenance & Traceability
`BEC-868954A0` Device Association (line 4561)	Reconciled predecessor — decomposed	BEC CT3: Device Utilisation Administration; BEC CT3: Device OA&M Metadata Management
`BEC-22B624C5` Device-Patient/Location Assignment (line 4577)	Retained Canonical	Feature: Device-Patient/Location Assignment
`BEC-DBE8DD8A` Device Identity Management (line 4588)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-D81970AA` Device State Management (line 4600)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-49A18E8C` Device Relationship Management (line 4612)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-FC971E86` Device Provenance Tracking (line 4624)	Refined / repositioned	Feature: Device Association Lineage
`BEC-6548F58A` Device Connectivity (line 4637)	Reconciled predecessor — consolidated	BEC CT3: Device OA&M Metadata Management
`BEC-45355492` Device Endpoint Registration (line 4653)	Retained Canonical	Feature: Device Endpoint Registration
`BEC-D0A68C93` Device Identity Management (line 4664)	Subsumed concern; no distinct Feature delta	BEC CT3: Device OA&M Metadata Management; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-B24562CE` Device State Management (line 4676)	Subsumed concern; no distinct Feature delta	BEC CT3: Device OA&M Metadata Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-14A1B631` Device Relationship Management (line 4688)	Subsumed concern; no distinct Feature delta	BEC CT3: Device OA&M Metadata Management; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`BEC-41EDE6C0` Device Provenance Tracking (line 4700)	Subsumed concern; no distinct Feature delta	BEC CT3: Device OA&M Metadata Management; Enterprise CT2: Provenance & Traceability

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-FF45E08F` Care Administration Enablement (line 4713)	Reconciled to human-reviewed hierarchy	BEC CT1: Care Administration Enablement
`BEC-DD7184DF` Client-Centred Administration (line 4735)	Retained Canonical responsibility	BEC CT2: Client-Centred Administration
`BEC-25DB15BA` Client Intake & Demographic Update (line 4745)	Retained Proposed responsibility	BEC CT3: Client Intake & Demographic Update
`BEC-4AB59D5A` Demographic Update Ingestion (line 4762)	Retained Canonical	Feature: Demographic Update Ingestion
`BEC-BDCCD5ED` Demographic Change Distribution (line 4773)	Retained Canonical	Feature: Demographic Change Distribution
`BEC-163E98AD` Context Establishment (line 4784)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Intake & Demographic Update; Enterprise CT2: Context Management
`BEC-80DFF5D9` State Monitoring (line 4794)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Intake & Demographic Update; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-F29D899B` Exception Management (line 4804)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Intake & Demographic Update; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-D55A3518` Information Retrieval (line 4814)	Subsumed concern; no distinct Feature delta	BEC CT3: Client Intake & Demographic Update; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-C16B2202` Referral Administration (line 4825)	Retained Canonical responsibility	BEC CT2: Referral Administration
`BEC-CD3F820D` Referral Intake & Progression (line 4835)	Retained Proposed responsibility	BEC CT3: Referral Intake & Progression
`BEC-8E9AD826` Referral Document Ingestion (line 4853)	Retained Canonical	Feature: Referral Document Ingestion
`BEC-2C8F0BD3` Referral Supporting Information (line 4864)	Retained Canonical	Feature: Referral Supporting Information
`BEC-6E903B7F` Referral Status & Outcome Tracking (line 4875)	Retained Canonical	Feature: Referral Status & Outcome Tracking
`BEC-7C40F1C9` Referral Lifecycle Management (line 4887)	Subsumed concern; no distinct Feature delta	BEC CT3: Referral Intake & Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-A551E3F4` Referral Routing (line 4899)	Refined / repositioned	Feature: Referral Destination Coordination
`BEC-C4E89E34` Referral Status Tracking (line 4911)	Subsumed concern; no distinct Feature delta	BEC CT3: Referral Intake & Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-57720BC2 Referral Outcome Association (line 4923)	Subsumed concern; no distinct Feature delta	BEC CT3: Referral Intake & Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-2ABE98E1 Scheduling Administration (line 4936)	Retained Canonical responsibility	BEC CT2: Scheduling Administration
BEC-8FEF95D4 Appointment & Booking Administration (line 4946)	Retained Proposed responsibility	BEC CT3: Appointment & Booking Administration
BEC-8C2F56E0 Appointment Notification Ingestion (line 4963)	Retained Canonical	Feature: Appointment Notification Ingestion
BEC-4143272F Appointment Status Synchronization (line 4974)	Retained Canonical	Feature: Appointment Status Synchronization
BEC-262653A0 Appointment Lifecycle Management (line 4985)	Subsumed concern; no distinct Feature delta	BEC CT3: Appointment & Booking Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-D9D79FA0 Participant Coordination (line 4997)	Subsumed concern; no distinct Feature delta	BEC CT3: Appointment & Booking Administration; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-425F9192 Availability Validation (line 5009)	Subsumed concern; no distinct Feature delta	BEC CT3: Appointment & Booking Administration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-4D29BEFF Appointment Exception Management (line 5021)	Subsumed concern; no distinct Feature delta	BEC CT3: Appointment & Booking Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-63E23D60 Episode & Encounter Administration (line 5034)	Retained Canonical responsibility	BEC CT2: Episode & Encounter Administration
BEC-1C528FC1 Encounter Lifecycle Management (line 5044)	Retained Proposed responsibility	BEC CT3: Encounter Lifecycle Management
BEC-0DC29356 Encounter Context Creation (line 5062)	Retained Canonical	Feature: Encounter Context Creation
BEC-5D8CA48C Encounter State Progression (line 5073)	Retained Canonical	Feature: Encounter State Progression
BEC-847FCFDB Encounter Bed/Location Association (line 5085)	Retained Canonical	Feature: Encounter Bed/Location Association
BEC-3EC99787 Encounter Lifecycle Management (line 5096)	Subsumed concern; no distinct Feature delta	BEC CT3: Encounter Lifecycle Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-46431EC5 Care Context Association (line 5108)	Subsumed concern; no distinct Feature delta	BEC CT3: Encounter Lifecycle Management; Enterprise CT2: Context Management
BEC-12EB85C6 Participant Association (line 5120)	Subsumed concern; no distinct Feature delta	BEC CT3: Encounter Lifecycle Management; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-D42C14E5` Encounter History Retrieval (line 5132)	Refined / repositioned	Feature: Encounter History Retrieval
`BEC-B6BE6E1D` Order Administration (line 5145)	Retained Canonical responsibility	BEC CT2: Order Administration
`BEC-5E9ECEA1` Closed-Loop Order Progression (line 5155)	Retained Proposed responsibility	BEC CT3: Closed-Loop Order Progression
`BEC-D0729BFB` Order Request Ingestion (line 5174)	Retained Canonical	Feature: Order Request Ingestion
`BEC-20BEDA79` Order Destination Resolution & Routing (line 5185)	Retained Canonical	Feature: Order Destination Resolution & Routing
`BEC-E57851C6` Order Closed-Loop Progression Tracking (line 5196)	Retained Canonical	Feature: Order Closed-Loop Progression Tracking
`BEC-84EFF3A3` Order Cancellation & Modification Coordination (line 5208)	Retained Canonical	Feature: Order Cancellation & Modification Coordination
`BEC-142E39C3` Request Lifecycle Management (line 5219)	Subsumed concern; no distinct Feature delta	BEC CT3: Closed-Loop Order Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-977D6A7D` Request Fulfilment Tracking (line 5231)	Subsumed concern; no distinct Feature delta	BEC CT3: Closed-Loop Order Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-81D316C2` Request Status Correlation (line 5243)	Subsumed concern; no distinct Feature delta	BEC CT3: Closed-Loop Order Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-AA804BEF` Request Exception Management (line 5255)	Subsumed concern; no distinct Feature delta	BEC CT3: Closed-Loop Order Progression; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-B67A0E82` Diagnostic Administration (line 5268)	Retained Canonical responsibility	BEC CT2: Diagnostic Administration
`BEC-C0DD4810` Diagnostic Workflow Management (line 5278)	Retained Proposed responsibility	BEC CT3: Diagnostic Workflow Management
`BEC-53E5C256` Diagnostic Request Correlation (line 5295)	Retained Canonical	Feature: Diagnostic Request Correlation
`BEC-4B72F961` Diagnostic Report Ingestion & Binding (line 5306)	Retained Canonical	Feature: Diagnostic Report Ingestion & Binding
`BEC-322A3126` Diagnostic Report Distribution (line 5317)	Retained Canonical	Feature: Diagnostic Report Distribution
`BEC-CAA7BA59` Diagnostic Result Association (line 5328)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Workflow Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-67FF7EC6 Diagnostic Status Tracking (line 5340)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Workflow Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-47552FD9 Diagnostic Provenance Capture (line 5352)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Workflow Management; Enterprise CT2: Provenance & Traceability
BEC-B4278279 Medication Administration (line 5365)	Retained Canonical responsibility	BEC CT2: Medication Administration
BEC-1EFAA849 Medication Order & Dispense Management (line 5375)	Retained Proposed responsibility	BEC CT3: Medication Order & Dispense Management
BEC-B26D4F17 Medication Order Ingestion (line 5397)	Retained Canonical	Feature: Medication Order Ingestion
BEC-0DE47133 Dispense Event Tracking (line 5408)	Retained Canonical	Feature: Dispense Event Tracking
BEC-D93D1C27 Medication Administration Record (MAR) Ingestion (line 5419)	Retained Canonical	Feature: Medication Administration Record (MAR) Ingestion
BEC-AB5055A8 Request Lifecycle Management (line 5430)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-E0ABA2EF Request Fulfilment Tracking (line 5442)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-D9110CE8 Request Status Correlation (line 5454)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-B0E3D03A Request Exception Management (line 5466)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-074E40CB Medication Identity Resolution (line 5478)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-AFA2A006 Medication Safety Context Validation (line 5490)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-9E3BA088 Medication Activity Tracking (line 5502)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-7D9A630B Medication Information Correlation (line 5514)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Order & Dispense Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-3233022C Procedure Administration (line 5527)	Retained Canonical responsibility	BEC CT2: Procedure Administration

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-489D3945 Procedure & Surgical Administration (line 5537)	Retained Proposed responsibility	BEC CT3: Procedure & Surgical Administration
BEC-F696528F Procedure Booking Ingestion (line 5552)	Retained Canonical	Feature: Procedure Booking Ingestion
BEC-75A8D392 Procedural Documentation Ingestion (line 5563)	Retained Canonical	Feature: Procedural Documentation Ingestion
BEC-5DBEFF67 Context Establishment (line 5574)	Subsumed concern; no distinct Feature delta	BEC CT3: Procedure & Surgical Administration; Enterprise CT2: Context Management
BEC-6100D197 State Monitoring (line 5584)	Subsumed concern; no distinct Feature delta	BEC CT3: Procedure & Surgical Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-82E3935C Exception Management (line 5594)	Subsumed concern; no distinct Feature delta	BEC CT3: Procedure & Surgical Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-C925BA63 Information Retrieval (line 5604)	Subsumed concern; no distinct Feature delta	BEC CT3: Procedure & Surgical Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-BF6253DC Clinical Record Administration (line 5615)	Retained Canonical responsibility	BEC CT2: Clinical Record Administration
BEC-0CE70156 Clinical Documentation Lifecycle (line 5625)	Retained Proposed responsibility	BEC CT3: Clinical Documentation Lifecycle Administration
BEC-096B53FA Clinical Document Ingestion (line 5641)	Retained Canonical	Feature: Clinical Document Ingestion
BEC-A9A0C02B Document Versioning & Supersession (line 5652)	Retained Canonical	Feature: Document Versioning & Supersession
BEC-2963C113 Document Metadata Indexing (line 5664)	Retained Canonical	Feature: Document Metadata Indexing
BEC-836351BA Context Establishment (line 5675)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Documentation Lifecycle Administration; Enterprise CT2: Context Management
BEC-CF085C02 State Monitoring (line 5685)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Documentation Lifecycle Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-A27F8453 Exception Management (line 5695)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Documentation Lifecycle Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-FDC92A6D Information Retrieval (line 5705)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Documentation Lifecycle Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-389062F6 Care Plan Administration (line 5716)	Retained Canonical responsibility	BEC CT2: Care Plan Administration
BEC-E263E2D1 Care Plan Coordination (line 5726)	Retained Proposed responsibility	BEC CT3: Care Plan Lifecycle & Coordination

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-E3837AB5 Care Plan Ingestion & Distribution (line 5741)	Retained Canonical	Feature: Care Plan Ingestion & Distribution
BEC-D4E51095 Care Goal Association (line 5752)	Refined / repositioned	Feature: Care Goal & Plan Progression Representation
BEC-6CCF8E04 Care Plan Lifecycle Tracking (line 5764)	Refined / repositioned	Feature: Care Goal & Plan Progression Representation
BEC-065E070B Care Plan Context Retrieval (line 5776)	Subsumed concern; no distinct Feature delta	BEC CT3: Care Plan Lifecycle & Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-0B71E5CB Clinical Communication Administration (line 5789)	Retained Canonical responsibility	BEC CT2: Clinical Communication Administration
BEC-6D84C2C1 Secure Clinical Communication (line 5799)	Retained Proposed responsibility	BEC CT3: Secure Clinical Communication
BEC-C55CEDBD Clinical Communication Distribution (line 5816)	Retained Canonical	Feature: Clinical Communication Distribution
BEC-357AFA04 Delivery Acknowledgement Tracking (line 5827)	Retained Canonical	Feature: Delivery Acknowledgement Tracking
BEC-4966229D Communication Audit Logging (line 5838)	Retained Canonical	Feature: Communication Audit Logging
BEC-BEE7FF8B Communication Context Management (line 5849)	Subsumed concern; no distinct Feature delta	BEC CT3: Secure Clinical Communication; Enterprise CT2: Context Management
BEC-B1E25071 Recipient Resolution (line 5861)	Subsumed concern; no distinct Feature delta	BEC CT3: Secure Clinical Communication; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-206D85C4 Communication Delivery Tracking (line 5873)	Subsumed concern; no distinct Feature delta	BEC CT3: Secure Clinical Communication; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-09D4A645 Funding & Coverage Administration (line 5886)	Retained Canonical responsibility	BEC CT2: Funding & Coverage Administration
BEC-47D10EB4 Clinical Charging & Claims Administration (line 5896)	Retained Canonical responsibility	BEC CT2: Clinical Charging & Claims Administration
BEC-3A4524B6 Clinical Service Context Enablement (line 5906)	Reconciled to human-reviewed hierarchy	BEC CT1: Clinical Service Dependency Management; BEC CT3: Primary Care Coordination; BEC CT3: Acute Care Monitoring & Event Coordination; BEC CT3: Emergency Care Progression; BEC CT3: Multidisciplinary Inpatient Care Coordination; BEC CT3: Diagnostic Service Integration; BEC CT3: Medication Therapy Provision and Monitoring; BEC CT3: Screening and Immunisation Enablement; BEC CT3: Community Care Coordination; BEC CT3: Outreach Care Continuity; BEC CT3: Remote Patient Monitoring

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-AEAB9E52 Clinical Assessment (line 5931)	Retained Canonical responsibility	BEC CT2: Clinical Assessment
BEC-14FD8EDD Primary Care (line 5941)	Retained Canonical responsibility	BEC CT3: Primary Care Coordination
BEC-2D7A6DDE Primary Care Coordination (line 5951)	Retained Canonical responsibility	BEC CT3: Primary Care Coordination
BEC-AB31E6BC General Practice Shared Record Ingestion (line 5968)	Retained Canonical	Feature: General Practice Shared Record Ingestion
BEC-E7F50E85 Primary Care Notification Dispatch (line 5979)	Retained Canonical	Feature: Primary Care Notification Dispatch
BEC-DB56036E Context Establishment (line 5990)	Subsumed concern; no distinct Feature delta	BEC CT3: Primary Care Coordination; Enterprise CT2: Context Management
BEC-8A07FDB4 Eligibility Validation (line 6000)	Subsumed concern; no distinct Feature delta	BEC CT3: Primary Care Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-CA466B56 Assignment & Routing (line 6010)	Subsumed concern; no distinct Feature delta	BEC CT3: Primary Care Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-A945B4F5 State Tracking (line 6020)	Subsumed concern; no distinct Feature delta	BEC CT3: Primary Care Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-2737B30C Exception Management (line 6030)	Subsumed concern; no distinct Feature delta	BEC CT3: Primary Care Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-F6FC078E Outcome Reporting (line 6040)	Subsumed concern; no distinct Feature delta	BEC CT3: Primary Care Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-4E6DB5F9 Acute Care (line 6051)	Retained Canonical responsibility	BEC CT3: Acute Care Monitoring & Event Coordination
BEC-0FAC6B5A Acute Care Monitoring & Event Coordination (line 6061)	Retained Canonical responsibility	BEC CT3: Acute Care Monitoring & Event Coordination
BEC-5DD077B5 Acute Clinical State Event Capture (line 6078)	Retained Canonical	Feature: Acute Clinical State Event Capture
BEC-870D4A16 Acute Care Clinical Context Provision (line 6089)	Retained Canonical	Feature: Acute Care Clinical Context Provision
BEC-3FB23504 Context Establishment (line 6100)	Subsumed concern; no distinct Feature delta	BEC CT3: Acute Care Monitoring & Event Coordination; Enterprise CT2: Context Management
BEC-CA660CB0 Eligibility Validation (line 6110)	Subsumed concern; no distinct Feature delta	BEC CT3: Acute Care Monitoring & Event Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-CAAB941D Assignment & Routing (line 6120)	Subsumed concern; no distinct Feature delta	BEC CT3: Acute Care Monitoring & Event Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-DFC60092 State Tracking (line 6130)	Subsumed concern; no distinct Feature delta	BEC CT3: Acute Care Monitoring & Event Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-1298D75B Exception Management (line 6140)	Subsumed concern; no distinct Feature delta	BEC CT3: Acute Care Monitoring & Event Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-8ED2F7E4 Outcome Reporting (line 6150)	Subsumed concern; no distinct Feature delta	BEC CT3: Acute Care Monitoring & Event Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-729DFC66 Emergency Care (line 6161)	Retained Canonical responsibility	BEC CT3: Emergency Care Progression
BEC-A9EBF8FE Emergency Care Progression (line 6171)	Retained Canonical responsibility	BEC CT3: Emergency Care Progression
BEC-3E1EE6B4 Emergency Department Arrival & Triage Ingestion (line 6186)	Retained Canonical	Feature: Emergency Department Arrival & Triage Ingestion
BEC-78F5EADB Emergency Care Cross-Facility Correlation (line 6197)	Retained Canonical	Feature: Emergency Care Cross-Facility Correlation
BEC-ED681415 Context Establishment (line 6208)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Care Progression; Enterprise CT2: Context Management
BEC-ADC2AC77 State Monitoring (line 6218)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Care Progression; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-0EC08E00 Exception Management (line 6228)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Care Progression; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-1723857C Information Retrieval (line 6238)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Care Progression; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-EBFE3FB1 Inpatient Care (line 6249)	Retained Canonical responsibility	BEC CT3: Multidisciplinary Inpatient Care Coordination
BEC-17776772 Multidisciplinary Inpatient Care Coordination (line 6259)	Retained Canonical responsibility	BEC CT3: Multidisciplinary Inpatient Care Coordination
BEC-6F99412C Inpatient Rounding Context Provision (line 6276)	Retained Canonical	Feature: Inpatient Rounding Context Provision
BEC-93C32399 Inpatient Clinical Progression Tracking (line 6287)	Retained Canonical	Feature: Inpatient Clinical Progression Tracking
BEC-826FE486 Subject Identity Correlation (line 6298)	Subsumed concern; no distinct Feature delta	BEC CT3: Multidisciplinary Inpatient Care Coordination; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-941ACC0E Subject Relationship Management (line 6310)	Subsumed concern; no distinct Feature delta	BEC CT3: Multidisciplinary Inpatient Care Coordination; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-152BD757 Subject Context Management (line 6322)	Subsumed concern; no distinct Feature delta	BEC CT3: Multidisciplinary Inpatient Care Coordination; Enterprise CT2: Context Management
BEC-C7242AB0 Subject Information Discovery (line 6334)	Subsumed concern; no distinct Feature delta	BEC CT3: Multidisciplinary Inpatient Care Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-69DD19FD Ambulatory Care (line 6347)	Retained Canonical responsibility	BEC CT2: Ambulatory Care
BEC-6C600AC1 Surgical & Procedural Care (line 6357)	Retained Canonical responsibility	BEC CT2: Surgical & Procedural Care
BEC-27E670BB Diagnostic Services (line 6367)	Retained Canonical responsibility	BEC CT3: Diagnostic Service Integration
BEC-BB867C0A Diagnostic Service Integration (line 6377)	Retained Canonical responsibility	BEC CT3: Diagnostic Service Integration
BEC-E7A7A26F Laboratory & Imaging Result Distribution (line 6398)	Retained Canonical	Feature: Laboratory & Imaging Result Distribution
BEC-821F8A39 Diagnostic History Consolidation (line 6409)	Retained Canonical	Feature: Diagnostic History Consolidation
BEC-48D54B88 Service Definition Management (line 6420)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Semantic Governance & Conformance
BEC-E8E1F920 Service Eligibility Management (line 6432)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-870CAEA8 Service Availability Management (line 6444)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-14C91D01 Service Discovery (line 6456)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-BDA9D14E Diagnostic Request Correlation (line 6468)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-A299B6D0 Diagnostic Result Association (line 6480)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-D4C02A64 Diagnostic Status Tracking (line 6492)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-FCD0D5E4 Diagnostic Provenance Capture (line 6504)	Subsumed concern; no distinct Feature delta	BEC CT3: Diagnostic Service Integration; Enterprise CT2: Provenance & Traceability
BEC-BFDB47CA Therapeutic Services (line 6517)	Retained Canonical responsibility	BEC CT2: Therapeutic Services
BEC-B48BC336 Medication Therapy (line 6527)	Retained Canonical responsibility	BEC CT3: Medication Therapy Provision and Monitoring
BEC-0789035C Medication Therapy Provision and Monitoring (line 6537)	Retained Canonical responsibility	BEC CT3: Medication Therapy Provision and Monitoring

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-D1549684` Medication History Reconciliation Ingestion (line 6554)	Retained Canonical	Feature: Medication History Reconciliation Ingestion
`BEC-95E0F2E9` Medication Administration Tracking (line 6565)	Retained Canonical	Feature: Medication Administration Tracking
`BEC-910194D7` Medication Identity Resolution (line 6576)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Therapy Provision and Monitoring; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`BEC-324D8BAD` Medication Safety Context Validation (line 6588)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Therapy Provision and Monitoring; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`BEC-0F34B38A` Medication Activity Tracking (line 6600)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Therapy Provision and Monitoring; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-B3D9C0F4` Medication Information Correlation (line 6612)	Subsumed concern; no distinct Feature delta	BEC CT3: Medication Therapy Provision and Monitoring; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-13B74601` Allied Health (line 6625)	Retained Canonical responsibility	BEC CT2: Allied Health
`BEC-8C53E055` Preventive Care (line 6635)	Retained Canonical responsibility	BEC CT3: Screening and Immunisation Enablement
`BEC-6ECB9608` Screening and Immunisation Enablement (line 6645)	Retained Canonical responsibility	BEC CT3: Screening and Immunisation Enablement
`BEC-21483404` Immunisation Event Submission (line 6662)	Retained Canonical	Feature: Immunisation Event Submission
`BEC-B609C8F5` Screening Recall Notification Distribution (line 6673)	Retained Canonical	Feature: Screening Recall Notification Distribution
`BEC-261578C2` Context Establishment (line 6684)	Subsumed concern; no distinct Feature delta	BEC CT3: Screening and Immunisation Enablement; Enterprise CT2: Context Management
`BEC-C577B7C8` State Monitoring (line 6694)	Subsumed concern; no distinct Feature delta	BEC CT3: Screening and Immunisation Enablement; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-F9613D20` Exception Management (line 6704)	Subsumed concern; no distinct Feature delta	BEC CT3: Screening and Immunisation Enablement; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-336AAF9D` Information Retrieval (line 6714)	Subsumed concern; no distinct Feature delta	BEC CT3: Screening and Immunisation Enablement; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-A8289AED` Rehabilitation (line 6725)	Retained Canonical responsibility	BEC CT2: Rehabilitation
`BEC-CDB89B75` Community Care (line 6735)	Retained Canonical responsibility	BEC CT3: Community Care Coordination
`BEC-50748227` Community Care Coordination (line 6745)	Retained Canonical responsibility	BEC CT3: Community Care Coordination

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-70EEF999` Community Encounter Ingestion (line 6762)	Retained Canonical	Feature: Community Encounter Ingestion
`BEC-5CB15F94` Community Care Plan Synchronization (line 6773)	Retained Canonical	Feature: Community Care Plan Synchronization
`BEC-D96C0908` Context Establishment (line 6784)	Subsumed concern; no distinct Feature delta	BEC CT3: Community Care Coordination; Enterprise CT2: Context Management
`BEC-A1012BD3` Eligibility Validation (line 6794)	Subsumed concern; no distinct Feature delta	BEC CT3: Community Care Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`BEC-5D53CB17` Assignment & Routing (line 6804)	Subsumed concern; no distinct Feature delta	BEC CT3: Community Care Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-52C8768A` State Tracking (line 6814)	Subsumed concern; no distinct Feature delta	BEC CT3: Community Care Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-A79A3ED2` Exception Management (line 6824)	Subsumed concern; no distinct Feature delta	BEC CT3: Community Care Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-1725AA08` Outcome Reporting (line 6834)	Subsumed concern; no distinct Feature delta	BEC CT3: Community Care Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-9DDD1BD3` Outreach Care (line 6845)	Retained Canonical responsibility	BEC CT3: Outreach Care Continuity
`BEC-B8EB379A` Outreach Care Continuity (line 6855)	Retained Canonical responsibility	BEC CT3: Outreach Care Continuity
`BEC-6A90478E` Outreach Visit Context Provision (line 6870)	Retained Canonical	Feature: Outreach Visit Context Provision
`BEC-8BCC3746` Outreach Encounter Outcome Ingestion (line 6880)	Retained Canonical	Feature: Outreach Encounter Outcome Ingestion
`BEC-89F15AEF` Context Establishment (line 6891)	Subsumed concern; no distinct Feature delta	BEC CT3: Outreach Care Continuity; Enterprise CT2: Context Management
`BEC-59BE4E9F` State Monitoring (line 6901)	Subsumed concern; no distinct Feature delta	BEC CT3: Outreach Care Continuity; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-8F38AE8D` Exception Management (line 6911)	Subsumed concern; no distinct Feature delta	BEC CT3: Outreach Care Continuity; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-607A7A67` Information Retrieval (line 6921)	Subsumed concern; no distinct Feature delta	BEC CT3: Outreach Care Continuity; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-F9E9DD8E` Remote & Virtual Care (line 6932)	Retained Canonical responsibility	BEC CT3: Remote Patient Monitoring
`BEC-68C27168` Remote Patient Monitoring (line 6942)	Retained Canonical responsibility	BEC CT3: Remote Patient Monitoring

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-FAC2E484 Remote Telemetry Ingestion (line 6959)	Retained Canonical	Feature: Remote Telemetry Ingestion
BEC-D4A0665C Virtual Care Session Context Binding (line 6970)	Retained Canonical	Feature: Virtual Care Session Context Binding
BEC-08D640DD Subject Identity Correlation (line 6981)	Subsumed concern; no distinct Feature delta	BEC CT3: Remote Patient Monitoring; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-2A678BE0 Subject Relationship Management (line 6993)	Subsumed concern; no distinct Feature delta	BEC CT3: Remote Patient Monitoring; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-0B235F68 Subject Context Management (line 7005)	Subsumed concern; no distinct Feature delta	BEC CT3: Remote Patient Monitoring; Enterprise CT2: Context Management
BEC-6B015BE0 Subject Information Discovery (line 7017)	Subsumed concern; no distinct Feature delta	BEC CT3: Remote Patient Monitoring; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-4FA35177 Health Service Operations Enablement (line 7030)	Reconciled to human-reviewed hierarchy	BEC CT1: Health Service Operations Enablement
BEC-ED6D98E8 Clinic & Practice Operations (line 7059)	Retained Canonical responsibility	BEC CT2: Clinic & Practice Operations
BEC-FD9982FA Session & Queue Coordination (line 7069)	Retained Proposed responsibility	BEC CT3: Session & Queue Coordination
BEC-C271D00D Clinic Session Tracking (line 7086)	Retained Canonical	Feature: Clinic Session Tracking
BEC-1D3DC25A Operational Queue Progression (line 7096)	Retained Canonical	Feature: Operational Queue Progression
BEC-F2DDA45C Context Establishment (line 7108)	Subsumed concern; no distinct Feature delta	BEC CT3: Session & Queue Coordination; Enterprise CT2: Context Management
BEC-2E5F2477 Eligibility Validation (line 7118)	Subsumed concern; no distinct Feature delta	BEC CT3: Session & Queue Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-91E57A4C Assignment & Routing (line 7128)	Subsumed concern; no distinct Feature delta	BEC CT3: Session & Queue Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-28A55B14 State Tracking (line 7138)	Subsumed concern; no distinct Feature delta	BEC CT3: Session & Queue Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-334D43B0 Exception Management (line 7148)	Subsumed concern; no distinct Feature delta	BEC CT3: Session & Queue Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-4734D5E1 Outcome Reporting (line 7158)	Subsumed concern; no distinct Feature delta	BEC CT3: Session & Queue Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-25E1A896 Ward Operations (line 7169)	Retained Canonical responsibility	BEC CT2: Ward Operations

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-CE9BB804` Ward Logistics Coordination (line 7179)	Retained Proposed responsibility	BEC CT3: Ward Logistics Coordination
`BEC-9408FFFA` Ward Patient Flow Coordination (line 7196)	Retained Canonical	Feature: Ward Patient Flow Coordination
`BEC-06C10C04` Clinical Handover Context Assembly (line 7206)	Retained Canonical	Feature: Clinical Handover Context Assembly
`BEC-0029BF2B` Context Establishment (line 7216)	Subsumed concern; no distinct Feature delta	BEC CT3: Ward Logistics Coordination; Enterprise CT2: Context Management
`BEC-616FFDC2` Eligibility Validation (line 7226)	Subsumed concern; no distinct Feature delta	BEC CT3: Ward Logistics Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`BEC-0DA17C6A` Assignment & Routing (line 7236)	Subsumed concern; no distinct Feature delta	BEC CT3: Ward Logistics Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-E6E33D5A` State Tracking (line 7246)	Subsumed concern; no distinct Feature delta	BEC CT3: Ward Logistics Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-E8384DB4` Exception Management (line 7256)	Subsumed concern; no distinct Feature delta	BEC CT3: Ward Logistics Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-4864C81B` Outcome Reporting (line 7266)	Subsumed concern; no distinct Feature delta	BEC CT3: Ward Logistics Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-389897E6` Theatre Operations (line 7277)	Retained Canonical responsibility	BEC CT2: Theatre Operations
`BEC-D248D868` Perioperative Logistics (line 7287)	Retained Proposed responsibility	BEC CT3: Perioperative Logistics Coordination
`BEC-98713ABB` Operating Theatre Case Progression (line 7302)	Retained Canonical	Feature: Operating Theatre Case Progression
`BEC-B1E9630D` Perioperative Resource Notification (line 7314)	Retained Canonical	Feature: Perioperative Resource Notification
`BEC-63DA723B` Context Establishment (line 7325)	Subsumed concern; no distinct Feature delta	BEC CT3: Perioperative Logistics Coordination; Enterprise CT2: Context Management
`BEC-9F573A4C` State Monitoring (line 7335)	Subsumed concern; no distinct Feature delta	BEC CT3: Perioperative Logistics Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-1233B732` Exception Management (line 7345)	Subsumed concern; no distinct Feature delta	BEC CT3: Perioperative Logistics Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-792B9999` Information Retrieval (line 7355)	Subsumed concern; no distinct Feature delta	BEC CT3: Perioperative Logistics Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-78F79EDB` Emergency Department Operations (line 7366)	Retained Canonical responsibility	BEC CT2: Emergency Department Operations

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-469419AB ED Operational Coordination (line 7376)	Retained Proposed responsibility	BEC CT3: Emergency Department Operational Coordination
BEC-2C9F0B1A ED Bed & Bay Tracking (line 7393)	Retained Canonical	Feature: ED Bed & Bay Tracking
BEC-A7E6EDD2 ED Length of Stay Tracking (line 7403)	Retained Canonical	Feature: ED Length of Stay Tracking
BEC-4CFE53A4 Context Establishment (line 7413)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Department Operational Coordination; Enterprise CT2: Context Management
BEC-67497215 Eligibility Validation (line 7423)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Department Operational Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-A2B49B7C Assignment & Routing (line 7433)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Department Operational Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-704E04EA State Tracking (line 7443)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Department Operational Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-BCE98F58 Exception Management (line 7453)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Department Operational Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-5E9192CC Outcome Reporting (line 7463)	Subsumed concern; no distinct Feature delta	BEC CT3: Emergency Department Operational Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-3EA18C92 Outpatient Operations (line 7474)	Retained Canonical responsibility	BEC CT2: Outpatient Operations
BEC-07398864 Outpatient Flow Enablement (line 7484)	Retained Proposed responsibility	BEC CT3: Outpatient Flow Coordination
BEC-4382A684 Outpatient Arrival Registration (line 7501)	Retained Canonical	Feature: Outpatient Arrival Registration
BEC-53EB5F3C Post-Clinic Order Coordination (line 7511)	Retained Canonical	Feature: Post-Clinic Order Coordination
BEC-0D49726E Subject Identity Correlation (line 7521)	Subsumed concern; no distinct Feature delta	BEC CT3: Outpatient Flow Coordination; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-FCB21DFC Subject Relationship Management (line 7533)	Subsumed concern; no distinct Feature delta	BEC CT3: Outpatient Flow Coordination; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-CD0D5A3A Subject Context Management (line 7545)	Subsumed concern; no distinct Feature delta	BEC CT3: Outpatient Flow Coordination; Enterprise CT2: Context Management
BEC-17DAEA1C Subject Information Discovery (line 7557)	Subsumed concern; no distinct Feature delta	BEC CT3: Outpatient Flow Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-DBA54D09` Bed & Care-Place Management (line 7570)	Retained Canonical responsibility	BEC CT2: Bed & Care-Place Management
`BEC-9F88F277` Bed State & Turnover Management (line 7580)	Retained Proposed responsibility	BEC CT3: Bed & Care-Place State Administration
`BEC-1A021F75` Bed Availability Tracking (line 7596)	Retained Canonical	Feature: Bed Availability Tracking
`BEC-ABDDBA4A` Bed State Progression (line 7607)	Retained Canonical	Feature: Bed State Progression
`BEC-CB31C6E4` Bed Readiness Notification (line 7619)	Retained Canonical	Feature: Bed Readiness Notification
`BEC-21F00612` Context Establishment (line 7630)	Subsumed concern; no distinct Feature delta	BEC CT3: Bed & Care-Place State Administration; Enterprise CT2: Context Management
`BEC-25AAE44B` State Monitoring (line 7640)	Subsumed concern; no distinct Feature delta	BEC CT3: Bed & Care-Place State Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-49BB68FA` Exception Management (line 7650)	Subsumed concern; no distinct Feature delta	BEC CT3: Bed & Care-Place State Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-BE0DEFDF` Information Retrieval (line 7660)	Subsumed concern; no distinct Feature delta	BEC CT3: Bed & Care-Place State Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-07A2FD29` Clinical Resource Management (line 7671)	Reconciled predecessor — superseded by reviewed capability	BEC CT3: Device Utilisation Administration; BEC CT1: Clinical Service Dependency Management
`BEC-556B4A3F` Specialized Equipment Coordination (line 7681)	Retained Proposed responsibility	BEC CT3: Device Utilisation Administration
`BEC-5B685445` Mobile Clinical Equipment Tracking (line 7697)	Retained Canonical	Feature: Mobile Clinical Equipment Tracking
`BEC-E7E5A452` Context Establishment (line 7708)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Context Management
`BEC-AA245FF0` Eligibility Validation (line 7718)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`BEC-A870EB50` Assignment & Routing (line 7728)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`BEC-F1B486E3` State Tracking (line 7738)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-99E2FA68` Exception Management (line 7748)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-4D2A47AC Outcome Reporting (line 7758)	Subsumed concern; no distinct Feature delta	BEC CT3: Device Utilisation Administration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-DB7874AF Service Capacity Management (line 7769)	Retained Canonical responsibility	BEC CT2: Service Capacity Management
BEC-D90FAB4C Facility Demand & Capacity Telemetry (line 7779)	Retained Proposed responsibility	BEC CT3: Service Demand & Capacity Coordination
BEC-A5CDE7AA Operational Capacity Metric Aggregation (line 7793)	Retained Canonical	Feature: Operational Capacity Metric Aggregation
BEC-3559F81B Context Establishment (line 7804)	Subsumed concern; no distinct Feature delta	BEC CT3: Service Demand & Capacity Coordination; Enterprise CT2: Context Management
BEC-F78FC8D3 State Monitoring (line 7814)	Subsumed concern; no distinct Feature delta	BEC CT3: Service Demand & Capacity Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-0203E495 Exception Management (line 7824)	Subsumed concern; no distinct Feature delta	BEC CT3: Service Demand & Capacity Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-A2BE39BF Information Retrieval (line 7834)	Subsumed concern; no distinct Feature delta	BEC CT3: Service Demand & Capacity Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-20C1E1B7 Clinical Workforce Management (line 7845)	Retained Canonical responsibility	BEC CT2: Clinical Workforce Capability Monitoring
BEC-F8C1B2DF Clinical Rostering (line 7855)	Retained Canonical responsibility	BEC CT2: Clinical Rostering
BEC-71740710 Mobile Staff Management (line 7865)	Retained Canonical responsibility	BEC CT2: Mobile Staff Management
BEC-EC43C4CB Mobile Worker Dispatch & Safety (line 7875)	Retained Proposed responsibility	BEC CT3: Mobile Worker Dispatch & Safety Coordination
BEC-D243C132 Mobile Worker Task Dispatch (line 7892)	Retained Canonical	Feature: Mobile Worker Task Dispatch
BEC-CAFF9E49 Mobile Visit Status Tracking (line 7903)	Retained Canonical	Feature: Mobile Visit Status Tracking
BEC-02254531 Context Establishment (line 7913)	Subsumed concern; no distinct Feature delta	BEC CT3: Mobile Worker Dispatch & Safety Coordination; Enterprise CT2: Context Management
BEC-04274FDD Eligibility Validation (line 7923)	Subsumed concern; no distinct Feature delta	BEC CT3: Mobile Worker Dispatch & Safety Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-BFBF8974 Assignment & Routing (line 7933)	Subsumed concern; no distinct Feature delta	BEC CT3: Mobile Worker Dispatch & Safety Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-E87B589F State Tracking (line 7943)	Subsumed concern; no distinct Feature delta	BEC CT3: Mobile Worker Dispatch & Safety Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-7FD6C955 Exception Management (line 7953)	Subsumed concern; no distinct Feature delta	BEC CT3: Mobile Worker Dispatch & Safety Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-D56786A8 Outcome Reporting (line 7963)	Subsumed concern; no distinct Feature delta	BEC CT3: Mobile Worker Dispatch & Safety Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-AA196190 On-Call Management (line 7974)	Retained Canonical responsibility	BEC CT2: On-Call Management
BEC-4BE71CB3 On-Call Directory Resolution (line 7984)	Retained Proposed responsibility	BEC CT3: On-Call Availability & Contact Resolution
BEC-722ABA6E Active On-Call Provider Resolution (line 7998)	Retained Canonical	Feature: Active On-Call Provider Resolution
BEC-4CFA3E90 Context Establishment (line 8009)	Subsumed concern; no distinct Feature delta	BEC CT3: On-Call Availability & Contact Resolution; Enterprise CT2: Context Management
BEC-703E878F State Monitoring (line 8019)	Subsumed concern; no distinct Feature delta	BEC CT3: On-Call Availability & Contact Resolution; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-E452ABBD Exception Management (line 8029)	Subsumed concern; no distinct Feature delta	BEC CT3: On-Call Availability & Contact Resolution; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-783C1ED2 Information Retrieval (line 8039)	Subsumed concern; no distinct Feature delta	BEC CT3: On-Call Availability & Contact Resolution; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-4A72D708 Work Allocation & Dispatch (line 8050)	Retained Canonical responsibility	BEC CT2: Work Allocation & Dispatch
BEC-09C6941C Operational Work Assignment & Supervision (line 8060)	Retained Proposed responsibility	BEC CT3: Operational Work Assignment & Dispatch Coordination
BEC-02438774 Work Item Instantiation (line 8079)	Retained Canonical	Feature: Work Item Instantiation
BEC-2A278A2C Worker Matching & Allocation (line 8090)	Retained Canonical	Feature: Worker Matching & Allocation
BEC-11BC936A Work Dispatch Delivery (line 8100)	Retained Canonical	Feature: Work Dispatch Delivery
BEC-4D7128D9 Work Progress Oversight & Escalation (line 8110)	Retained Canonical	Feature: Work Progress Oversight & Escalation
BEC-59CF5F93 Context Establishment (line 8122)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Work Assignment & Dispatch Coordination; Enterprise CT2: Context Management
BEC-7B45C0B7 Eligibility Validation (line 8132)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Work Assignment & Dispatch Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-AD553DA5 Assignment & Routing (line 8142)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Work Assignment & Dispatch Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-D5AEDF90 State Tracking (line 8152)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Work Assignment & Dispatch Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-C536BD61 Exception Management (line 8162)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Work Assignment & Dispatch Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-A3268ADD Outcome Reporting (line 8172)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Work Assignment & Dispatch Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-06CF5B6D Clinical Training & Education (line 8183)	Retained Canonical responsibility	BEC CT2: Clinical Training & Education
BEC-DE805931 Clinical Competency Management (line 8193)	Retained Canonical responsibility	BEC CT2: Clinical Competency Administration
BEC-9C5388A1 Clinical Credential Management (line 8203)	Retained Canonical responsibility	BEC CT2: Clinical Qualification Administration
BEC-E3152A69 Credential Verification Checkpoint (line 8213)	Retained Proposed responsibility	BEC CT3: Provider Clinical Qualification Tracking
BEC-5CD4669F Operational Credential Check (line 8227)	Retained Canonical	Feature: Operational Credential Check
BEC-D3D46BF9 Context Establishment (line 8237)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Context Management
BEC-E9331A31 State Monitoring (line 8247)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-9736C9DB Exception Management (line 8257)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-3DE5AC43 Information Retrieval (line 8267)	Subsumed concern; no distinct Feature delta	BEC CT3: Provider Clinical Qualification Tracking; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-D401771A Clinical Supervision (line 8278)	Retained Canonical responsibility	BEC CT2: Clinical Supervision
BEC-55FBCC7C Patient Transport (line 8288)	Retained Canonical responsibility	BEC CT2: Patient Transport
BEC-854FF619 Patient Transfer Coordination (line 8298)	Retained Proposed responsibility	BEC CT3: Patient Transfer Coordination
BEC-EC853A0F Transport Request Ingestion (line 8315)	Retained Canonical	Feature: Transport Request Ingestion
BEC-58DCCBDB Transport Dispatch & Progress Tracking (line 8326)	Retained Canonical	Feature: Transport Dispatch & Progress Tracking

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-05915061 Subject Identity Correlation (line 8338)	Subsumed concern; no distinct Feature delta	BEC CT3: Patient Transfer Coordination; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-F93AE731 Subject Relationship Management (line 8350)	Subsumed concern; no distinct Feature delta	BEC CT3: Patient Transfer Coordination; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-CBF19A4E Subject Context Management (line 8362)	Subsumed concern; no distinct Feature delta	BEC CT3: Patient Transfer Coordination; Enterprise CT2: Context Management
BEC-A0E56E8C Subject Information Discovery (line 8374)	Subsumed concern; no distinct Feature delta	BEC CT3: Patient Transfer Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-BB578270 Clinical Logistics Coordination (line 8387)	Retained Canonical responsibility	BEC CT2: Clinical Logistics Coordination
BEC-999FB8AB Specimen & Supply Courier Tracking (line 8397)	Retained Proposed responsibility	BEC CT3: Specimen Movement Traceability
BEC-0BDCFA64 Pathology Specimen Transport Tracking (line 8412)	Retained Canonical	Feature: Pathology Specimen Transport Tracking
BEC-900A97F0 Specimen Identity Management (line 8424)	Subsumed concern; no distinct Feature delta	BEC CT3: Specimen Movement Traceability; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
BEC-9950BA59 Specimen Custody Tracking (line 8436)	Subsumed concern; no distinct Feature delta	BEC CT3: Specimen Movement Traceability; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Provenance & Traceability
BEC-E08C24F3 Specimen Relationship Management (line 8448)	Subsumed concern; no distinct Feature delta	BEC CT3: Specimen Movement Traceability; Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
BEC-3C941BB0 Discharge Management (line 8461)	Retained Canonical responsibility	BEC CT2: Discharge Management
BEC-3414F1EB Discharge Progression & Coordination (line 8471)	Retained Proposed responsibility	BEC CT3: Discharge Progression & Coordination
BEC-B778D440 Discharge Milestone Tracking (line 8489)	Retained Canonical	Feature: Discharge Milestone Tracking
BEC-F3326F02 Discharge Readiness & Coordination Oversight (line 8500)	Retained Canonical	Feature: Discharge Readiness & Coordination Oversight
BEC-8B11D01C Discharge Notification Dispatch (line 8512)	Retained Canonical	Feature: Discharge Notification Dispatch
BEC-377B8814 Context Establishment (line 8523)	Subsumed concern; no distinct Feature delta	BEC CT3: Discharge Progression & Coordination; Enterprise CT2: Context Management
BEC-54362543 Eligibility Validation (line 8533)	Subsumed concern; no distinct Feature delta	BEC CT3: Discharge Progression & Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-92074CE1 Assignment & Routing (line 8543)	Subsumed concern; no distinct Feature delta	BEC CT3: Discharge Progression & Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-D2873E18 State Tracking (line 8553)	Subsumed concern; no distinct Feature delta	BEC CT3: Discharge Progression & Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-AF97D3B7 Exception Management (line 8563)	Subsumed concern; no distinct Feature delta	BEC CT3: Discharge Progression & Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-D56CF336 Outcome Reporting (line 8573)	Subsumed concern; no distinct Feature delta	BEC CT3: Discharge Progression & Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-03705603 Shared Information and Coordination Enablement (line 8584)	Reconciled to human-reviewed hierarchy	BEC CT1: Shared Information & Coordination Enablement; BEC CT1: Health Informatics Management; BEC CT1: Independent Assurance Enablement
BEC-97605881 Patient Clinical Record (line 8605)	Retained Canonical responsibility	BEC CT2: Patient Clinical Record
BEC-15AB949F Longitudinal Clinical Record Governance (line 8615)	Retained Proposed responsibility	BEC CT3: Longitudinal Clinical Record Administration
BEC-B5EBE9A9 Longitudinal Health Record (LHR) Assembly (line 8633)	Retained Canonical	Feature: Longitudinal Health Record (LHR) Assembly
BEC-81C53E17 Active Clinical Record Access (line 8644)	Retained Canonical	Feature: Active Clinical Record Access
BEC-A048190C Durable Clinical Record Preservation (line 8655)	Retained Canonical	Feature: Durable Clinical Record Preservation
BEC-9F1C5969 Record Context Association (line 8666)	Subsumed concern; no distinct Feature delta	BEC CT3: Longitudinal Clinical Record Administration; Enterprise CT2: Context Management
BEC-3149DD0E Record Provenance Management (line 8678)	Refined / repositioned	Feature: Information Transformation Lineage
BEC-67C43D3D Record Lifecycle Management (line 8690)	Subsumed concern; no distinct Feature delta	BEC CT3: Longitudinal Clinical Record Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-F9431119 Record Discovery (line 8702)	Subsumed concern; no distinct Feature delta	BEC CT3: Longitudinal Clinical Record Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-14BBAAFD HIE Integration Services (line 8715)	Retained Canonical responsibility	BEC CT2: Health Information Exchange Enablement
BEC-0FADE98F Multi-Model Information Exchange (line 8725)	Retained Proposed responsibility	BEC CT3: HIE Implementation Management
BEC-A4404077 Direct Submission Ingestion & Delivery (line 8743)	Retained Canonical	Feature: Direct Submission Ingestion & Delivery

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-3110386A` Clinical Record Retrieval Mediation (line 8754)	Retained Canonical	Feature: Clinical Record Retrieval Mediation
`BEC-E1974228` Addressed Clinical Distribution (line 8765)	Retained Canonical	Feature: Addressed Clinical Distribution
`BEC-05FAD383` Event-Driven Syndication (line 8776)	Retained Canonical	Feature: Event-Driven Syndication
`BEC-E86070E3` Information Validation (line 8787)	Subsumed concern; no distinct Feature delta	BEC CT3: HIE Implementation Management; Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
`BEC-9924A266` Information Correlation (line 8797)	Subsumed concern; no distinct Feature delta	BEC CT3: HIE Implementation Management; Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
`BEC-C048B88A` Information Retrieval (line 8807)	Subsumed concern; no distinct Feature delta	BEC CT3: HIE Implementation Management; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-F497FE5F` Information Provenance (line 8817)	Subsumed concern; no distinct Feature delta	BEC CT3: HIE Implementation Management; Enterprise CT2: Provenance & Traceability
`BEC-90EED6CF` Information Lifecycle Management (line 8827)	Subsumed concern; no distinct Feature delta	BEC CT3: HIE Implementation Management; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-0EE16B9D` Health Information Access (line 8838)	Retained Canonical responsibility	BEC CT2: Health Information Access
`BEC-73452901` Governed Query & Search (line 8848)	Retained Proposed responsibility	BEC CT3: Governed Health Information Retrieval
`BEC-AB8EDD15` Federated Clinical Query (line 8863)	Retained Canonical	Feature: Federated Clinical Query
`BEC-4601C816` Access-Controlled Information Filtering (line 8874)	Retained Canonical	Feature: Access-Controlled Information Filtering
`BEC-72A9140C` Context Establishment (line 8885)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Health Information Retrieval; Enterprise CT2: Context Management
`BEC-474C9605` State Monitoring (line 8895)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Health Information Retrieval; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`BEC-5604785C` Exception Management (line 8905)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Health Information Retrieval; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`BEC-C138A5D7` Information Retrieval (line 8915)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Health Information Retrieval; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-4553B47B` Health Information Communication (line 8926)	HUMAN ADJUDICATION REQUIRED	BEC CT3: Health Information Communication
`BEC-763876A9` Boundary Protocol Mediation (line 8936)	HUMAN ADJUDICATION REQUIRED	BEC CT3: Health Information Communication

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-0DFE1C63 Standards-Based Boundary Exchange (line 8950)	Retained Canonical	Feature: Standards-Based Boundary Exchange
BEC-8EDC75DD Context Establishment (line 8961)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Information Communication; Enterprise CT2: Context Management
BEC-C8A17F6A State Monitoring (line 8971)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Information Communication; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-6F511B71 Exception Management (line 8981)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Information Communication; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-2EBB9F24 Information Retrieval (line 8991)	Subsumed concern; no distinct Feature delta	BEC CT3: Health Information Communication; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-203E2D62 Health Information Control (line 9002)	Retained Canonical responsibility	BEC CT2: Health Information Control
BEC-18459CCD Policy & Security Governance (line 9012)	Retained Canonical responsibility	BEC CT2: Health Information Control
BEC-E140C81D Health Information Access & Consent Control (line 9029)	Retained Canonical	Feature: Health Information Access & Consent Control
BEC-CA2EF6F6 Security Context Propagation (line 9040)	Retained Canonical	Feature: Security Context Propagation
BEC-0BBA26D3 Non-PHI Compliance Auditing (line 9051)	Retained Canonical	Feature: Non-PHI Compliance Auditing
BEC-9EB61778 Criteria Management (line 9062)	Subsumed concern; no distinct Feature delta	BEC CT2: Health Information Control; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-E3405F10 Evidence Association (line 9072)	Subsumed concern; no distinct Feature delta	BEC CT2: Health Information Control; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-5BAB5EA0 Policy Evaluation (line 9082)	Subsumed concern; no distinct Feature delta	BEC CT2: Health Information Control; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-1D2CA7D5 Decision Traceability (line 9092)	Refined / repositioned	Feature: Mandatory Significant Activity Audit Capture
BEC-CB8F5161 Exception Reporting (line 9102)	Subsumed concern; no distinct Feature delta	BEC CT2: Health Information Control; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-B622BACF Clinical Knowledge Services (line 9113)	Retained Canonical responsibility	BEC CT2: Clinical Knowledge Services
BEC-43B6CE64 Terminology & Reference Mapping (line 9123)	Retained Proposed responsibility	BEC CT3: Governed Reference Dataset Administration
BEC-B0351502 Clinical Concept Resolution (line 9139)	Retained Canonical	Feature: Clinical Concept Resolution
BEC-DC869393 Bidirectional Terminology Mapping (line 9150)	Retained Canonical	Feature: Bidirectional Terminology Mapping

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-77A76C7C Information Validation (line 9161)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Reference Dataset Administration; Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
BEC-CBEC5F4E Information Correlation (line 9171)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Reference Dataset Administration; Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
BEC-E5775112 Information Retrieval (line 9181)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Reference Dataset Administration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-A5D9596D Information Provenance (line 9191)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Reference Dataset Administration; Enterprise CT2: Provenance & Traceability
BEC-760AE591 Information Lifecycle Management (line 9201)	Subsumed concern; no distinct Feature delta	BEC CT3: Governed Reference Dataset Administration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-4CDE0BAB Clinical Collaboration (line 9212)	Retained Canonical responsibility	BEC CT2: Clinical Collaboration
BEC-0C37BAFF LHR-Aware Multidisciplinary Collaboration (line 9222)	Retained Proposed responsibility	BEC CT3: Clinical Context Collaboration
BEC-84D2C1DF Care-Team Clinical Collaboration Coordination (line 9238)	Retained Canonical	Feature: Care-Team Clinical Collaboration Coordination
BEC-0B2CF342 In-Conversation LHR Query Resolution (line 9249)	Retained Canonical	Feature: In-Conversation LHR Query Resolution
BEC-24657506 Zero-PHI Collaboration Metadata Governance (line 9260)	Retained Canonical	Feature: Zero-PHI Collaboration Metadata Governance
BEC-3F3E28AF Context Establishment (line 9271)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Context Collaboration; Enterprise CT2: Context Management
BEC-911087B0 State Monitoring (line 9281)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Context Collaboration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-2DF10AA6 Exception Management (line 9291)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Context Collaboration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-0CD2C755 Information Retrieval (line 9301)	Subsumed concern; no distinct Feature delta	BEC CT3: Clinical Context Collaboration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-E1807E93 General Collaboration (line 9312)	Retained Canonical responsibility	BEC CT2: General Collaboration
BEC-2D46A392 Operational Messaging (line 9322)	Retained Proposed responsibility	BEC CT3: Operational Collaboration
BEC-1631787F Operational Team Chat Enablement (line 9336)	Retained Canonical	Feature: Operational Team Chat Enablement
BEC-074F43D0 Context Establishment (line 9347)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Collaboration; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-D75B5D46 State Monitoring (line 9357)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Collaboration; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-5368E298 Exception Management (line 9367)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Collaboration; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-7D688C5A Information Retrieval (line 9377)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational Collaboration; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-0C4A6874 Workflow & Activity Coordination (line 9388)	HUMAN ADJUDICATION REQUIRED	BEC CT3: Workflow & Activity Coordination
BEC-A3E2C598 Multi-Work-Unit Orchestration (line 9398)	HUMAN ADJUDICATION REQUIRED	BEC CT3: Workflow & Activity Coordination
BEC-36C99CCB Work Order Progression (line 9417)	Retained Canonical	Feature: Work Order Progression
BEC-4306DDD8 To Do Decision & Approval Coordination (line 9429)	Retained Canonical	Feature: To Do Decision & Approval Coordination
BEC-782CD2A4 Synthetic Task Orchestration (line 9441)	Retained Canonical	Feature: Synthetic Task Orchestration
BEC-76B22B19 Workflow Timeout & Escalation Oversight (line 9453)	Retained Canonical	Feature: Workflow Timeout & Escalation Oversight
BEC-A1649A9C Context Establishment (line 9464)	Subsumed concern; no distinct Feature delta	BEC CT3: Workflow & Activity Coordination; Enterprise CT2: Context Management
BEC-2078C8D9 Eligibility Validation (line 9474)	Subsumed concern; no distinct Feature delta	BEC CT3: Workflow & Activity Coordination; Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
BEC-9FFA6F7F Assignment & Routing (line 9484)	Subsumed concern; no distinct Feature delta	BEC CT3: Workflow & Activity Coordination; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
BEC-D25D58D7 State Tracking (line 9494)	Subsumed concern; no distinct Feature delta	BEC CT3: Workflow & Activity Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-642BD633 Exception Management (line 9504)	Subsumed concern; no distinct Feature delta	BEC CT3: Workflow & Activity Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-6B53749D Outcome Reporting (line 9514)	Subsumed concern; no distinct Feature delta	BEC CT3: Workflow & Activity Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-78F5DB56 Calendar Management (line 9525)	Retained Canonical responsibility	BEC CT2: Calendar Management
BEC-8CD7EA17 Operational & Clinical Calendar Sync (line 9535)	Retained Proposed responsibility	BEC CT3: Operational & Clinical Calendar Coordination
BEC-79108DF3 Schedule Projection Integration (line 9550)	Retained Canonical	Feature: Schedule Projection Integration

Prior concept / source line	Decision — Proposed	Current responsibility / reference
BEC-45628C87 Temporal Event Correlation (line 9561)	Retained Canonical	Feature: Temporal Event Correlation
BEC-9F5E17D1 Context Establishment (line 9572)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational & Clinical Calendar Coordination; Enterprise CT2: Context Management
BEC-922D6FFF State Monitoring (line 9582)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational & Clinical Calendar Coordination; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-76D61F9A Exception Management (line 9592)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational & Clinical Calendar Coordination; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-1022E17A Information Retrieval (line 9602)	Subsumed concern; no distinct Feature delta	BEC CT3: Operational & Clinical Calendar Coordination; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-6AA02A96 Presentation Services (line 9613)	Retained Canonical responsibility	BEC CT2: Presentation Services
BEC-2DCF9553 Decoupled Human Interaction & Dashboards (line 9623)	Retained Proposed responsibility	BEC CT3: Contextual Information Presentation
BEC-06AB86FA Longitudinal Clinical & Operational Presentation (line 9637)	Retained Canonical	Feature: Longitudinal Clinical & Operational Presentation
BEC-53CDA4E5 Context Establishment (line 9648)	Subsumed concern; no distinct Feature delta	BEC CT3: Contextual Information Presentation; Enterprise CT2: Context Management
BEC-7CB4FD80 State Monitoring (line 9658)	Subsumed concern; no distinct Feature delta	BEC CT3: Contextual Information Presentation; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
BEC-D8AF8AC4 Exception Management (line 9668)	Subsumed concern; no distinct Feature delta	BEC CT3: Contextual Information Presentation; Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
BEC-DA832DCE Information Retrieval (line 9678)	Subsumed concern; no distinct Feature delta	BEC CT3: Contextual Information Presentation; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
BEC-233904DA Information Design Governance (line 9689)	Retained Canonical responsibility	BEC CT2: Information Design Governance
BEC-453A5B71 Canonical Semantics & Schema Governance (line 9699)	Retained Proposed responsibility	BEC CT3: Enterprise Canonical/Reusable Model Design
BEC-D57B8126 Information Standards & Semantic Governance (line 9715)	Retained Canonical	Feature: Information Standards & Semantic Governance
BEC-D881E00E Semantic Conformance Verification (line 9726)	Retained Canonical	Feature: Semantic Conformance Verification
BEC-74F7B6A8 Information Validation (line 9737)	Subsumed concern; no distinct Feature delta	BEC CT3: Enterprise Canonical/Reusable Model Design; Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`BEC-3B311E84` Information Correlation (line 9747)	Subsumed concern; no distinct Feature delta	BEC CT3: Enterprise Canonical/Reusable Model Design; Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
`BEC-E7F9D13F` Information Retrieval (line 9757)	Subsumed concern; no distinct Feature delta	BEC CT3: Enterprise Canonical/Reusable Model Design; Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`BEC-E383206B` Information Provenance (line 9767)	Subsumed concern; no distinct Feature delta	BEC CT3: Enterprise Canonical/Reusable Model Design; Enterprise CT2: Provenance & Traceability
`BEC-915C9E8F` Information Lifecycle Management (line 9777)	Subsumed concern; no distinct Feature delta	BEC CT3: Enterprise Canonical/Reusable Model Design; Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management

35.3. Enterprise

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`EC-BBAD8E2C` Entity and Identity (line 9792)	Integrated reusable responsibility	Enterprise CT1: Governed Information Operations
`EC-C0A97A83` Identity and Correlation (line 9803)	Integrated reusable responsibility	Enterprise CT2: Managed Entity & Relationship
`EC-461B8F05` Identity and Correlation Enablement (line 9813)	Integrated reusable responsibility	Enterprise CT2: Managed Entity & Relationship
`EC-1A42AFEF` Identity Correlation (line 9832)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`EC-00688388` Identity Management (line 9840)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`EC-13BDD6AE` Device Identity Management (line 9848)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`EC-157BAA94` Subject Identity Correlation (line 9856)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`EC-47108AAC` Information Correlation (line 9864)	Reclassified recurrence evidence	Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
`EC-EA771B95` Diagnostic Request Correlation (line 9872)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`EC-039334FA` Location Identity Management (line 9880)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
`EC-5FC97327` Medication Identity Resolution (line 9888)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`EC-D444A2C1` Medication Information Correlation (line 9896)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`EC-D418A306` Request Status Correlation (line 9904)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`EC-BAC316E4` Relationship and Structure (line 9913)	Integrated reusable responsibility	Enterprise CT2: Managed Entity & Relationship
`EC-F50F53E6` Relationship and Structure Enablement (line 9923)	Integrated reusable responsibility	Enterprise CT2: Managed Entity & Relationship
`EC-81917406` Device Relationship Management (line 9942)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-20119D58` Subject Relationship Management (line 9950)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-0B8C4EDC` Organisational Relationship Management (line 9958)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-49166B2A` Organisational Structure Management (line 9966)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-7A2DF974` Diagnostic Result Association (line 9974)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`EC-279FE89F` Evidence Association (line 9982)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`EC-A179D91B` Location Hierarchy Management (line 9990)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-366AFC9B` Professional Relationship Management (line 9998)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-E2F0CB40` Qualification Association (line 10006)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-F160E33E` Relationship Management (line 10014)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
`EC-47C59623` Information and Semantics (line 10023)	Integrated reusable responsibility	Enterprise CT1: Governed Information Operations
`EC-260111DC` Information Lifecycle (line 10035)	Integrated reusable responsibility	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`EC-AE5E9793` Information Lifecycle Enablement (line 10045)	Integrated reusable responsibility	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`EC-54C1BCAB` Information Retrieval (line 10062)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
`EC-9788F116` Subject Information Discovery (line 10070)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control

Prior concept / source line	Decision — Proposed	Current responsibility / reference
EC-BFA64AAF Information Correlation (line 10078)	Reclassified recurrence evidence	Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
EC-A3A099AC Information Lifecycle Management (line 10086)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-10E4EA11 Information Provenance (line 10094)	Reclassified recurrence evidence	Enterprise CT2: Provenance & Traceability
EC-23381315 Information Validation (line 10102)	Reclassified recurrence evidence	Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
EC-53DFF234 Medication Information Correlation (line 10110)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
EC-720E493E Request Lifecycle Management (line 10118)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-2A3844C6 Discovery and Retrieval (line 10127)	Integrated reusable responsibility	Enterprise CT2: Search & Discovery
EC-299D0172 Discovery and Retrieval Enablement (line 10137)	Integrated reusable responsibility	Enterprise CT2: Search & Discovery
EC-F2753584 Information Retrieval (line 10153)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
EC-82149096 Service Discovery (line 10161)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
EC-13D8A141 Subject Information Discovery (line 10169)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
EC-50A2C171 Organisation Discovery (line 10177)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
EC-009A4A5A Location Discovery (line 10185)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
EC-09A3798F Medication Identity Resolution (line 10193)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Search & Discovery
EC-26353D17 Practitioner Discovery (line 10201)	Reclassified recurrence evidence	Enterprise CT2: Search & Discovery; Enterprise CT2: Policy & Control
EC-89E071D0 Semantic Conformance (line 10210)	Integrated reusable responsibility	Enterprise CT2: Semantic Governance & Conformance
EC-47BECE6F Semantic Conformance Enablement (line 10220)	Integrated reusable responsibility	Enterprise CT2: Semantic Governance & Conformance
EC-63E0E334 Eligibility Validation (line 10233)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
EC-A86D200D Information Validation (line 10241)	Reclassified recurrence evidence	Enterprise CT2: Semantic Governance & Conformance; Enterprise CT2: Context Management
EC-93E36E5B Medication Safety Context Validation (line 10249)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
EC-DB69DAF0 Qualification Association (line 10257)	Reclassified recurrence evidence	Enterprise CT2: Managed Entity & Relationship; Enterprise CT2: Managed State & Lifecycle
EC-B80F9118 Context, Activity and Exchange (line 10266)	Integrated reusable responsibility	Enterprise CT1: Coordinated Activity & Interaction; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
EC-ADB90088 Context and State (line 10277)	Integrated reusable responsibility	Enterprise CT2: Context Management; Enterprise CT2: Managed State & Lifecycle
EC-6F46C125 Context and State Enablement (line 10287)	Integrated reusable responsibility	Enterprise CT2: Context Management; Enterprise CT2: Managed State & Lifecycle
EC-C3C11291 Context Establishment (line 10307)	Reclassified recurrence evidence	Enterprise CT2: Context Management
EC-CC8047E5 State Monitoring (line 10315)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-D122BC94 State Tracking (line 10323)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-FF26979C Service Availability Management (line 10331)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-6DCE8859 Device State Management (line 10339)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-1900212B Subject Context Management (line 10347)	Reclassified recurrence evidence	Enterprise CT2: Context Management
EC-53072EDD No Established Feature Decomposition (Adjacent / Reference Context) (line 10355)	Removed non-element placeholder	Non-element placeholder
EC-7D34D652 Diagnostic Status Tracking (line 10363)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
EC-582FEC53 Location Availability Management (line 10371)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-2739EBA1 Medication Safety Context Validation (line 10379)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
EC-DB0E7812 Request Status Correlation (line 10387)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
EC-8C0E52A8 Activity Coordination (line 10396)	Integrated reusable responsibility	Enterprise CT2: Activity & Execution
EC-D05CE29D Activity Coordination Enablement (line 10406)	Integrated reusable responsibility	Enterprise CT2: Activity & Execution
EC-5EE5F09E Assignment & Routing (line 10421)	Reclassified recurrence evidence	Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
EC-E762705E State Tracking (line 10429)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
EC-7F4BEB56 Device Provenance Tracking (line 10437)	Reclassified recurrence evidence	Enterprise CT2: Provenance & Traceability
EC-95E97505 Diagnostic Status Tracking (line 10445)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution

Prior concept / source line	Decision — Proposed	Current responsibility / reference
`EC-67A2E372` Medication Activity Tracking (line 10453)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`EC-A191A2BC` Request Fulfilment Tracking (line 10461)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Interoperability & Exchange; Enterprise CT2: Activity & Execution
`EC-8B8BC538` Governance, Trust and Assurance (line 10470)	Integrated reusable responsibility	Enterprise CT1: Trust, Accountability & Assurance
`EC-D9AF0B6F` Governed Control (line 10482)	Integrated reusable responsibility	Enterprise CT2: Policy & Control
`EC-E37E56AC` Governed Control Enablement (line 10492)	Integrated reusable responsibility	Enterprise CT2: Policy & Control
`EC-5948BFA2` Eligibility Validation (line 10504)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`EC-D98B7AAE` Service Eligibility Management (line 10512)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`EC-840C09A8` Policy Evaluation (line 10520)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`EC-DF0619B8` Provenance and Accountability (line 10529)	Integrated reusable responsibility	Enterprise CT2: Provenance & Traceability
`EC-3987B790` Provenance and Accountability Enablement (line 10539)	Integrated reusable responsibility	Enterprise CT2: Provenance & Traceability
`EC-E41C93C7` Device Provenance Tracking (line 10553)	Reclassified recurrence evidence	Enterprise CT2: Provenance & Traceability
`EC-313E85AB` Information Provenance (line 10561)	Reclassified recurrence evidence	Enterprise CT2: Provenance & Traceability
`EC-829F0BF7` Decision Traceability (line 10569)	Refined / repositioned	Feature: Mandatory Significant Activity Audit Capture
`EC-E9AA136D` Diagnostic Provenance Capture (line 10577)	Reclassified recurrence evidence	Enterprise CT2: Provenance & Traceability
`EC-7233F33E` Evidence Association (line 10585)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management
`EC-74BB4AD4` Assurance and Resilience (line 10594)	Integrated reusable responsibility	Enterprise CT2: Operational Assurance; Enterprise CT2: Service Guardian
`EC-B0D5A003` Assurance and Resilience Enablement (line 10604)	Integrated reusable responsibility	Enterprise CT2: Operational Assurance; Enterprise CT2: Service Guardian
`EC-50BE2841` Exception Management (line 10619)	Reclassified recurrence evidence	Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`EC-0F5671F1` State Monitoring (line 10627)	Reclassified recurrence evidence	Enterprise CT2: Managed State & Lifecycle; Enterprise CT2: Information Management
`EC-09035A01` Outcome Reporting (line 10635)	Reclassified recurrence evidence	Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`EC-20985EE7` Exception Reporting (line 10643)	Reclassified recurrence evidence	Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
`EC-34E6439F` Medication Safety Context Validation (line 10651)	Reclassified recurrence evidence	Enterprise CT2: Policy & Control; Enterprise CT2: Context Management

Prior concept / source line	Decision — Proposed	Current responsibility / reference
EC-85D9E54B Request Exception Management (line 10659)	Reclassified recurrence evidence	Enterprise CT2: Operational Assurance; Enterprise CT2: Activity & Execution
EC-1F33C0E8 Existing Enterprise Capabilities – Retained for Reconciliation (line 10668)	Integrated reusable responsibility	Enterprise CT1: Governed Information Operations; Enterprise CT1: Coordinated Activity & Interaction; Enterprise CT1: Trust, Accountability & Assurance
EC-3BFF62CA EC-01 Managed Entity & Relationship (line 10689)	Retained Canonical	Enterprise CT2: Managed Entity & Relationship
EC-5DE7A127 EC-02 Context Management (line 10701)	Retained Canonical	Enterprise CT2: Context Management
EC-D9B04B9B EC-03 Managed State & Lifecycle (line 10713)	Retained Canonical	Enterprise CT2: Managed State & Lifecycle
EC-BDDA8FD7 EC-04 Information Management (line 10725)	Retained Canonical	Enterprise CT2: Information Management
EC-C8C13214 EC-05 Search & Discovery (line 10737)	Retained Canonical	Enterprise CT2: Search & Discovery
EC-22EFEBD8 EC-06 Policy & Control (line 10749)	Retained Canonical	Enterprise CT2: Policy & Control
EC-23C71F1A EC-07 Provenance & Traceability (line 10761)	Retained Canonical	Enterprise CT2: Provenance & Traceability
EC-CE98D4FC EC-08 Interoperability & Exchange (line 10773)	Retained Canonical	Enterprise CT2: Interoperability & Exchange
EC-B5BCE57C EC-09 Event & Subscription (line 10785)	Retained Canonical	Enterprise CT2: Event & Subscription
EC-29FC1D73 EC-10 Activity & Execution (line 10797)	Retained Canonical	Enterprise CT2: Activity & Execution
EC-850DDBAA EC-11 Interaction & Experience (line 10809)	Retained Canonical	Enterprise CT2: Interaction & Experience
EC-E822F3AE EC-12 Operational Assurance (line 10821)	Retained Canonical	Enterprise CT2: Operational Assurance
EC-E7D5C47D EC-13 Semantic Governance & Conformance (line 10833)	Retained Canonical	Enterprise CT2: Semantic Governance & Conformance
EC-18C84D64 EC-14 Service Guardian (line 10845)	Retained Canonical	Enterprise CT2: Service Guardian

36. Source and derivation provenance

The frozen v0.1 source transcriptions retain their exact governed wording, including unresolved statements that Step 2 proposes to resolve structurally. The corpus register fingerprints current material surveyed for broader context. Citation is evidence, not a change to source standing. The v0.3 candidate is explicitly authorised prior reasoning, not authority. No superseded legacy architecture is imported.

Source	Fingerprint / role
docs/markdown/01-motivation/README.md	48017d61e7c11b438fd418d1e8ae547a4199e096273e0c7355299a69f1333cd8 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/drivers-assessments/assessments.md	038713e13714b8e306578f5f55cdcd3ee3f4663ed74359c5754d4a851184b19e Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/drivers-assessments/drivers.md	49617cdde51b4dc0d97924ec56b081e1e32784d5b59bb0e6e7c45ec3408ec025 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/goals-outcomes/business-outcomes.md	f1e29ecf0f8f48047edbd980a2b7876e99d21d67b40451e043ee7ee3f2c3b562 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/goals-outcomes/strategic-goals.md	52a10df1eba7c6ed81dd797f66288481da3268e344574183f08378ea012556a8 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/orientation-view.md	d51b08a35c0d248dc9b7cd260e89bda6ea8fc7234e3fdf335ecd9b4db141247b Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/principles/architectural-axioms.md	a1b50be8574dc032147a71b4176c770c4a2a3f7d63bb5b7111435748bfa77fd2 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/principles/reclassified-principles.md	eeb1a268f017faee4066b4195f1d0a151e021cb2c08bbed4d04556569c4165e7 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/requirements-constraints/external-constraints.md	49d58a4d09298428402edda60491ebd835ab2af33aa7c101045bce9ef07ba907 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/requirements-constraints/foundational-requirements.md	dc361c38927e1d90710d451f9af9203add2456f20553176a69dd055985e2ef7f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/requirements-constraints/master-requirements-catalogue.md	ffae8388c7953e8dc77862de03866eddb067795881e2698c4b6da0835b179fc2 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/reviews/independent-assurance-reconciliation.md	7f4c530156985532a1e6426891b330cae709568cd50eacf0478af6b32d40f276 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/stakeholders/enterprise-stakeholders.md	04283275d49dd9e8cb6e96c13a7b7a86ceec165fd341ce2f3e36a4beda4d4d25 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/01-motivation/stakeholders/external-authorities.md	d89dd970d9833fa5fc593abb2301e79e7f3af9fecacbc39f4b8c2d8a153aead6 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/README.md	962c0f5f3bba0449e08b3632072e26003d07046c10e7bfb90ca90f913f9a1bc2 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capabilities/business-capabilities.md	27d86f8a5efe789083c464cf3f5026d6ddab86ad8d207d94dd57807fd9cdd2c0 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capabilities/business-enabling-capabilities.md	8a7229be3343e027ab87c5421dec514d3803b80da254157be4e9d8c981010d49 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capabilities/enterprise-capabilities.md	4b887f8486827d26649b76d5d8ce3ef37ab9d11e861dbc6915ceb6202d92b2fb Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capabilities/ict-foundation-lenses.md	4e403ed83f4421687b7f794783d21e07b651d2db50d63246b1c39e2b26949bd9 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capabilities/index.md	1121f427162ebda0d2dbebeb0d5907ce982dfcc3a0aac5f85cab98c9722351ca Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capability-maps/capability-tier-model.md	186d9f1c6b10ce462db3214a423602f4ae9b69ce872d23b150dc68225c8b098a Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capability-maps/health-service-assurance-derivation.md	76bfe725b77fdc65409acf073cc516c193b49c1371fe758f4dae02192deda6d6 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/capability-maps/index.md	e740ea9caa80bc8bcbe7dfc7973c3008a1e50388908de415678383b17550f8c6 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/courses-of-action/index.md	24a8df5164864b81f7a3fa18763767aac4654045eace187cb605df17ad8cb950 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/courses-of-action/strategic-courses-of-action.md	dfc56c3a6de3284ecabc273032d9d8a3dabd9d41e01b29776e142a3aa0fd79f0 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/resources/index.md	a638b0f16a03613f19c604c95b0f1a4e503d16c462cecfbc19b82f5c80328a5e Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/resources/strategic-resources.md	99d4d512319d2f31e3724700f60de2f9bc595f21265d12471c970989150dad77 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/reviews/ax05-state-responsibility-reconciliation.md	01481b48ab72b398e2a045101b47b1b2f95967e3c7d8ceee4ad9177b0a0e96f9 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/reviews/business-capability-five-region-reconciliation.md	a46cfdb769cd27dc55a38a7ebf165a267f322ad6590910862d11e68de2b00e5d Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/reviews/fixed-capability-decomposition-decision.md	3e2e0545d22b6bdcd6a397e63048f7d5ba7896fca8c11b89863e9d0328a49d6a Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/strategic-views/index.md	85b5185502c9b6f5ca50e7890091ec7dfbaf4da46b5a955382fcec195ad589ff Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/strategic-views/logical-component-responsibilities.md	88679d22e32fe1883b2ef882dfe0ee897ce1c101a4b36ae1d5a2614ab035c944 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/02-strategy/strategic-views/strategic-value-streams.md	e35fc8a438c8b236e269e9aa8b071be492691b0c6446855e16e09f28c058e14a Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/README.md	8f38e40c40ca9124f3ad5c8bb700a4b7478f44710b8d3c94fdf75889d8628070 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/actors-roles/actors.md	67bb617bb5a0c717a16862575003d8e96c66e2113bc2da5aeec7b6b30ed1e365 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/actors-roles/roles.md	b9485c7b870badf8a8a4a6568b97721065ebbe4a9fb7057f578bad400f181c53 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/01-entity-management.md	15903e877addfbbc5fdc256377f48d52b7d7ac18c7b4f040baa8e8361666c69e Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/02-service-administration.md	7a26c3ad1d0581f6b5321b6089b99299e891256d5a739d5c074de306d9321878 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/03-service-delivery.md	9da2e94a389c6d075f13ddf5aaa1d1e0c54477fc973fb9763c93a6e95e5a86c9 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/04-health-service-operations.md	9de0e4725c772f96dc1a37ed9f668876cd7c3eca8aff56ae7f1865f39d54035d Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/05-intrinsic-enablement.md	80c699f9a5903c280c8a05fc87062b24a1c6ea2dea8843a92c5b53281d37ac8a Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/health-service-assurance.md	fa976de7417f4979c95f3d44cd2cbed979407744053918cc903e0d1059e4c5bb Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/behaviours/index.md	e672f554709a8ec2b3d9e7f053d9eb06d5ff21ece31fe8ef4c2c6210af8758be Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/collaborations-interactions/collaborations.md	d09f596259120814a00a1c1f567a5eb4626bbd3cf7b8e0e8a19c5bdf8b93324f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/collaborations-interactions/interactions.md	a1c77a1577e379345ed0b31e432d86939670a5b752d5886b1fb4fc95696e0de7 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/dependencies/cross-capability-dependencies.md	d536a30ec60c603107797acf7e2fcd803a91e8cf24bfafa342c58258c2110b39 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/information-responsibility/information-responsibility.md	2a51b1d1160337a181109c01225e8c8a2b464a7401af0d779c70664f4383a500 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/metamodel/business-architecture-metamodel.md	3b0906eb6d51aea98e17b3f6b31250e68c2aa7320a1989f2386449909ce8f19a Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/processes/business-processes.md	d7fd20fa50c3f41eb8a1d23b83ee6057eeb2f3225157f8677bdfdb3b478e23c2 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/03-business-architecture/reference-model/README.md	d78734031b7589f17394dde8405b0f23b4b842b2210b1b7dbb6718afb3299d09 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/README.md	9570e52eb6ab03654de91333104752462ec7c68cf626ab8dc6299f8dcac861c9 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/assemblies-views/assemblies-and-views.md	fff12df26e371344a251f8d79107203995893e8affb98c4fe935917d8b086126 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/governance/authority-custody-provenance.md	38e35b499cf60bb20698bb722febcc8e0d42a96fc39382a5243b4e21da010556 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/governance/information-lifecycle.md	b91f8be41247a11d833d0ccf9d6e0d53e72a7ceef02e20f01e54c4e0b3fbf29b Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/guardrails/modelling-guardrails.md	41905c955e611780bbbd3627b8f0203c24a3f5796a265079bf689ef64b79c73a Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/guardrails/observable-information-and-domain-meaning.md	13f18cb20d24592ce2394b4d0d26ed51479e55af4bd04383deb2adb639a5cae7 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/README.md	e738ec28847c9b52a73725e89eabebb16b2bb798663af688026d29f190b4f577 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/appointment-scheduling.md	d35dd97cad595ba05230873c47ded0851fc7a03091aded0a6e0bd3b00bd5d743 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/device.md	5c1646a522d91cdc32610c4b29b81159643b7bb439b4e318428798c4e686adc2 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/episode-encounter.md	b369d65ae7e2ba7f37bda2683ed879080f64532543c1b08f61d283768b4371d5 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/healthcare-location.md	53d37ab3061f507341b02042b59cbfbff23875db258efb8ef3467bc3c2c9d605 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/healthcare-service.md	ea164a82844b8589b11eef33e75957293239aea5362d1ffcb404e1031736cabd Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/order.md	e4db4f5158972f55af48f12a1e9be048437c95600b343823bb4bc9530b258a81 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/organisation.md	682648f0da0081bcfc7d4b76f0690598476720bdcde1531d50fe917cc39ef24f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/person-healthcare-subject.md	2b38456bb9c260cdc4f9113563cf3ba06e80c6ac2db8ead89a7200dc3af1fb39 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/practitioner.md	86df0038ecefcebd6bac4cb7cf58cdc015fa4c5033ad5468ad7511c693d88f47 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/referral.md	250e1d66fb517147632c7f6d1177000cb96c51def1a9299b7333593720433fa7 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/information-families/task-work.md	db2047ac5a673b00d908241eb0094e32c9856ed75e2be5b43939a5f940d0cc77 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/metamodel/information-architecture-metamodel.md	2eadaa3191cea51d9aade7acd99d8a8ee0a2cf114d847a3e87790476d531563f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/patterns/containment-and-collections.md	d97364cf3889d7e3c54860e8fea9803f93493998623b09c9dc28e8e7ec95bede Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/patterns/definition-to-accountability.md	a4ea759457bcdc2d75821c3149f1dc6fa7f4254a8c4a3e8c0de9e7cbc6f6d1b3 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/patterns/information-relationships.md	5071f45f26a1b588eada99449de61bbcac381addfa40f5f669961f5d73c61b65 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/patterns/metabole.md	a37bd64841d0d54542592f0f160a199de10c616b22360b0d88ed0e815cb9f676 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/patterns/search-results.md	a48f1ff1df015c17c6736860441ab9e026ace4667c39102c904bfe01c0d5c2d8 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/reviews/dokimasia-assurance-architectural-finding.md	0f2a39bf9a8bc690a9d2192a57d1b03f396ec249732867d06b854c3b3f35333c Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/reviews/dokimasia-r1-motivation-traceability-ax05-investigation.md	8c3ee826298f299887cb4e3c16c4c9bfeccac263027c777f1d02a96dc2f5d887 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/reviews/package2-g1-review.md	d5fde989273d26aa0823c66f50b7dd28e1eeaca180ed097b535c044c4a2d4b38 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/reviews/package2-g2-block1-review.md	fdde6724cadd31a13ec5da19ee855ea3043245536b5079c519f8a3b27b98108f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/04-information-architecture/traceability/domain03-traceability.md	221fa30288570fa58c554c40236a91be92d07aaf1348ecd123e3e0bbe649af08 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/README.md	ea4f256f57fbe0d5ca0667c3b45608ce98d23ea56b7f0eb168742ed509d8c8d9 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/agora-omilia-collaboration.md	49ee8d088dc0a75dd3dc7f85051f438d8cfe147ccaeaba20f24db38d9819348b Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/calliope-models-and-datasets.md	fe3cb785c610a0ea1360b0de6b0bd54c156f7dd2bf7e5ef4dc13561305408e87 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/candidate-feature-map.md	0585ca8887145ca21832bf1fcbf0f1829d1201251c7d7f68055b116bdf4afaab Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/candidate-system-map.md	b03ee4798ea1d4dbc1af67eec36fbdca6b74640e768a06c7d10850bbd485aea1 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/component-register.md	7f493dae1db3e77fe0342f7c21501763a4af2b4de9c59871a109626eaaad5aa0 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/context-and-persistence-management.md	4d2f4cd1890ad51e74fb87e75749ee6edb233b8e2e4fd28d9f00c852e21dd986 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/data-and-persistence.md	8f1b4222a8e1303001235366a17de7555e56141711d5599c6851a7d865cb2dcd Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Agora-Controlled-Collaboration-and-Omilia-Realisation.md	e111bd19156b160fe666bb3031ea5315e7d42e7752ff7e1337a511f9719673fa Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Calliope-Model-DataSet-and-Nomenclature-Management.md	8c7e835d3cc457ac8f0e755f74b171716e947ea5b9e7a689a853bf572378adeb Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Digital-Twin-Behaviour-and-Execution-Semantics.md	0a99f554246a94877b72c63b2b53d5c39b5b84e1b63404aabf18efab73ddfa4d Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Energeia-DigitalTwin-Dokimasia-Specialisation.md	3447f29389d698fe7a14025f4fb80c473891fc355e593cbcada3e5be2d2c5c01 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Energeia-Operational-Management-and-Praxis-Responsibility.md	3a87876d1c510877c18bff26467b5cf92f446087ba006698006580abf1c14c30 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Identity-Authentication-and-Authorisation-Architecture.md	22cb431cda966d7670104379d6211f8a7e08c5166a1982fba9876636748ec623 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Kleio-Audit-Information-Management-and-Resilient-Capture.md	0726432d82356eaa9a461c2e20f178a1172468d98d953508c1a0e19178843f82 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Kleio-Audit-Information-Management.md	423367e57e5e453e854826b7f46d14c4ba6159c68284f763bb18956b0c720a4b Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Mneme-Context-and-Pragma-Management.md	c5a2123aed81690d2eb13b6f51641db24041691474991d1e976ffe6f9cd9996f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Mneme-Publication-Subscription-and-Petasos-Handoff-Architecture.md	6b594bafb17aa5fbfaead3f33064ddfd9a350a6f08a5332ec0dabe38af9091f6 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Mnemosyne-Persistence-Management-and-Authoritative-Persistence-Oversight.md	c3f11e6363eea7c0120ac0d3bca1a08e1bde416608269529d7319982a07f3b50 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Pathways-Journeys-and-Assurance-Representation.md	d9a183290c3334fb7209e6841f351d790f54aa01979baf1c63f3f81a08d598e9 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Petasos-Durable-Message-Event-Queue-Management.md	e988799d8335314f26ecee6073230dfeed155049d2e376944dd77a58523480b8 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Pragma-Management-Responsibility-and-Energeia-Boundary.md	895247a2c7e31c5149aafbecd520cbab8635411aec9a41dc0c78fe20ca55ea90 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Pragma-Outputs-and-Pathway-Continuation.md	34aeaecd40a3e76469096d07ac67976e255e206a190c410b033b95369c95cfb3 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Pylai-External-Interoperability-Architecture.md	df49b5f2f5b26e2625a220ac35a58390dd9e94fe64ba485ef4de51c0e2a182cc Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/design-notes/DesignNote-Themis-Authorisation-Model-and-Distributed-Policy-Enforcement.md	5c757503d988e6de441cc4c9d4c434d5ca09acd009662a042eee0131aa229934 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/gaps.md	988534e6295d3b2437b49756c36fabb15d315885248b85a84e34a23a0dec66d4 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/middleware.md	7ee1a4ab1c24abcb052daa5f0392270a8dc4997dd4083db8fea19736bc363213 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/pathways-journeys-and-handoff.md	a397dee9209f308b7a6a9615ce0a46780faa6acc4724c0e537cf9e08894ab69c Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/pylai-external-interoperability.md	7d8efac8456b8659b1d57b4657e3372c0f6cb161f9b97dc0c5dc48bba1fc7836 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/reviews/b1-method-reconciliation.md	6e7c940097c1dfd0b71e139c079a578cc8f95285b419d2f02b101cbf965d2696 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/sources.md	f9cc7af3ab31f4cde0fa196198475d977f2f1b69d758dd33499ff70e9ceff176 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/silloge-binary-content-management.md	1716f02c2fc2545e9735e5e300de0d279958361030f6991e6183a82513d2244f Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/traceability/b2-1-individual-care-delivery-health-information-access.md	1e3b59e0b5d1168e75244528540f1c64084391f72551ba9fd375a70a83016ab1 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/traceability/health-information-access-worked-example.md	8f9ed6d63ef80b121d374b980e873cb5cf9cafe2742dc4735e4a497d5d00f003 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/05-information-systems-architecture/traceability/step-b-method.md	1561acdaca15a32bac1e3b4e7b3c052b07dc61f877793f8e1344c040f6b0d60e Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/governance/README.md	6bf8bc915d3b591ee406f17f11bc4f7cabdd6cbe264f94cf5f5a61066319fa74 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/governance/architectural-axioms.md	d0544fcb437b1ae43ed5fda6318ab4ab69a2e4ec614f73b34e8227b659b040c2 Current architectural corpus; source standing and unresolved matters remain applicable

Source	Fingerprint / role
docs/markdown/scratchpad/capability-reference-candidate/Harmonia-Capability-Reference-Candidate-v0.3.tex	82df3717855f1fc7572cd40bf60d81d23fc36c7567d304f3eb0b137ac8800d6f Explicitly authorised prior reasoning, reconciled concept by concept; not architectural authority

Source	Fingerprint / role
docs/latex-rad/Harmonia-EA-Capability-Map/v0.2-draft/reviewed-bec-hierarchy.json	208c6cbae846386e664067f1ac4798bd194209eb2098d54af98ed88dd294cc94 Explicit human-reviewed task input; reviewed candidate, not RADS acceptance

Part VI.

Governed RAD semantic detail

37. Canonical Business Capability Model (18 CT1 Domains)

RAD source: `docs/markdown/02-strategy/capabilities/business-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Business Capability Model (18 CT1 Domains)

Overview & Purpose

Business Capabilities describe **what the health enterprise/business must be capable of doing**. They are organisational abilities independent of technology, products, departments, professions or automation. This catalogue is the canonical R1/R2 reconciliation of the adjudicated Business Capability decisions recorded in DN-BC-001 through DN-BC-037. The scratchpad retains working reasoning only; the definitions, explicit tier/parent assertions and boundaries here govern the reconciled model. Provisional and unresolved matters are not promoted as approved hierarchy.

Business Enabling Capabilities describe **reusable enabling abilities Harmonia provides or supports**. The two hierarchies are distinct:

RAD source listing (presentation only; no inferred ancestry).

> Business Capability: CT1 → CT2 → CT3
>
> enabled-by / supported-by
>
> Business Enabling Capability: CT1 → CT2 → CT3 → Feature

The relationship is enablement/support, never decomposition or implementation equivalence. **Business Capabilities do not decompose into Harmonia Features.** Features are ultimately framed by Business Enabling CT3s. This catalogue establishes no new BC↔BEC mapping and does not reframe existing Features. Previously approved B2.1 and assurance support relationships retain their bounded authority without being extended to the new BC descendants.

Technology-sounding names such as Health Information Exchange, Digital Communication Channel, Identity Assurance and Access Control Enforcement describe organisational abilities. Physical, procedural and digital mechanisms illustrate exercise/enablement, not software-component identities. An interpretation requiring a particular software product to explain a BC is suspect. Where BC and BEC concern the same subject, their names must distinguish organisational ability from system enablement where reuse would imply equivalence or ownership. The existing Exchange name collision is explicitly unresolved below.

R1/R2 Scope and Decomposition Rules

> Capability decomposition SHALL be sufficient to establish Harmonia's architectural context and derivation requirements. It SHALL NOT pursue exhaustive decomposition of healthcare-enterprise capabilities where additional detail does not materially affect Harmonia responsibility, information semantics, interoperability, security, workflow enablement, traceability or another Harmonia architectural concern.
>
> Scope may determine decomposition depth. Scope must not distort decomposition semantics.

The stopping test asks whether distinguishing another ability would materially change how Harmonia behaves, governs, represents, exchanges, secures, coordinates or traces something. Once a branch is decomposed, its materially distinct peers must be coherent from the business perspective, independently of Harmonia's implementation interest. This does not require identical depth, a Feature for every peer or a complete healthcare-enterprise taxonomy. An undecomposed CT1/CT2 does not authorise a skipped tier or an invented CT3.

The metamodel governs tier meaning. Explicit CT1/CT2/CT3 and parent statements below establish the stated BC hierarchy; display ordinals, indentation and regional membership alone do not. CT4 and deeper tiers are outside scope. No ordinal is an allocated local code or Canonical ID. BEC tiers/ancestry remain separately unresolved under K9. Absent decomposition does not deny real-world abilities or assign them to an existing node.

Capability Modelling Quality Rules

Rule	Requirement
CM-R01 Coverage	Preserve the eighteen broad enterprise domains and five regions; decomposition is bounded by R1/R2 semantic sufficiency rather than exhaustive enterprise detail.
CM-R02 Orthogonality	Preserve distinct responsibilities, outcomes and authority even where capabilities share evidence, context or implementation machinery.
CM-R03 Semantic Clarity	Use clear business meanings; examples do not redefine capabilities as technology.
CM-R04 No Junk Drawers	Keep cohesive purposes; do not use residual categories to hide materially distinct abilities.
CM-R05 Global Name Uniqueness	Capability names are globally unique across the architecture. The unresolved BC/BEC Exchange collision is reported; neither identity is merged or renamed here.

Capability decomposition and educational documentation

Decomposition follows architectural distinction, not explanatory detail. Definition, state change, boundary, illustrative activities, care contexts and exclusions explain capabilities without establishing more children. History-taking, diagnosis, prescribing, medication administration, surgery, therapy and counselling are examples unless an explicit adjudication establishes a Capability identity.

B2-R05 — Capability Is Independent of Care Delivery Context

Setting, delivery mode, temporal pattern and organisational arrangement do not by themselves establish separate Business Capabilities.

B2-R06 — Context Independence Test

If changing setting, mode or time context does not materially change the business ability, it remains the same Capability. Acute, Planned/Scheduled and Longitudinal Care are not Individual Care Delivery's CT2 decomposition. Inpatient, ambulatory, community, home, virtual, emergency and elective remain illustrative contexts.

Harmonia Relevance Taxonomy

Classification	Meaning
Reference	Enterprise context without material effect on Harmonia responsibilities or processing obligations.
Adjacent	Harmonia consumes, represents or supplies contextual information without material participation in execution or coordination.
Harmonia-Relevant	Harmonia materially enables, connects, routes, coordinates or supplies information used in performing the capability.
Harmonia-Core	Harmonia provides substantive direct platform responsibility for the capability within its assigned boundary.

An established classification has exactly one of these values. Unresolved is a status, not a fifth value. CT1 relevance describes material participation across its bounded scope; it does not classify every descendant identically or confer enterprise-domain authority. Descendant relevance remains unallocated unless explicitly established. Contextual resource/state consumption alone warrants neither Core nor automatic Relevant classification. AX-18 preserves originating and domain authority. A Harmonia-Core enabling responsibility may support a Harmonia-Relevant BC without Harmonia owning that business activity.

The 18 CT1 Business Capability Domains

Five natural regions group the eighteen CT1s. Regions are contextual views, not capability ancestors, organisational boundaries or application ownership. They are distinct from the five BEC contextual views. All eighteen named catalogue domains below are explicitly **CT1 roots**; each branch statement explicitly establishes its CT2 parent and each table its CT3 parent.

Catalogue ordinal	CT1 Business Capability	Region	Relevance
01	Individual Care Delivery	Care & Health Delivery	Harmonia-Relevant
02	Care Access & Coordination	Care & Health Delivery	Harmonia-Relevant
03	Health Rights, Advocacy & Participation	Care & Health Delivery	Adjacent
04	Diagnostic, Therapeutic & Clinical Support Services	Care & Health Delivery	Harmonia-Relevant
05	Health Products & Clinical Technology	Care & Health Delivery	Harmonia-Relevant
06	Clinical Quality, Safety & Improvement	Care & Health Delivery	Harmonia-Relevant
07	Community Health & Wellbeing	Care & Health Delivery	Harmonia-Relevant
08	Population Health & Health-System Planning	Care & Health Delivery	Harmonia-Relevant
09	Health Information & Knowledge Management	Health Information & Digital Health	Harmonia-Core
10	Standards, Semantics & Reference Governance	Health Information & Digital Health	Harmonia-Core
11	Connected Health Services	Health Information & Digital Health	Harmonia-Core
12	Security, Privacy & Digital Trust	Health Information & Digital Health	Harmonia-Core
13	Health Research & Innovation	Research & Innovation	Harmonia-Relevant
14	Health Service Direction & Stewardship	Health Service Management	Harmonia-Relevant

Catalogue ordinal	CT1 Business Capability	Region	Relevance
15	Workforce & Organisational Capability	Health Service Management	Harmonia-Relevant
16	Corporate Resources & Enterprise Services	Health Service Management	Harmonia-Relevant
17	Health Service Governance	Governance & Assurance	Unresolved
18	Health Service Assurance	Governance & Assurance	Harmonia-Relevant

Region 1: Care & Health Delivery

01. Individual Care Delivery

Tier and ancestry: CT1 root.

Definition: Assess, clinically determine, plan, intervene, monitor and review individual care independently of setting, mode, time pattern or organisational arrangement.

Classification: Harmonia-Relevant.

The six CT2s are Individual Health Assessment, Clinical Determination, Individual Care Planning, Clinical Intervention, Individual Health Monitoring and Individual Care Review. Their semantic state changes do not prescribe execution order.

01.01 Individual Health Assessment

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to establish an understanding of an individual's current health state through the collection, consideration and evaluation of relevant clinical, health and contextual information.

State Change: `Health information → Assessed health state / findings`

Boundary: Assessment establishes the assessed health state or findings. Existing records can contribute, while direct examination, observation, history-taking or measurement may establish information independently of Health Information Access.

Illustrative Activities: History taking, physical examination, consideration of available clinical records and measurement of current observations.

Example Care Contexts: An emergency presentation, a scheduled consultation or a home visit may exercise the same assessment ability.

Explicit Exclusions: Establishing clinical conclusions, selecting the care response and performing an intervention are not the assessment itself.

01.02 Clinical Determination

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to interpret assessed health information and establish clinically meaningful conclusions about an individual's health state, condition, risk or prognosis.

State Change: `Assessed findings → Clinically interpreted understanding`

Boundary: Clinical Determination interprets findings and establishes their clinical meaning, including the clinical significance of change identified by monitoring.

Illustrative Activities: Diagnosis, clinical risk interpretation and prognostic judgement.

Example Care Contexts: Interpretation during acute care or a longitudinal follow-up consultation remains the same business ability.

Explicit Exclusions: Clinical Determination does not include establishing care needs or selecting the response to the determination.

01.03 Individual Care Planning

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to establish the intended clinical response to an individual's assessed and clinically interpreted health state, including the objectives, interventions and monitoring required to progress their care.

State Change: `Clinically interpreted understanding → Intended course of care`

Boundary: Planning establishes what is intended to happen. It does not imply that the planned care has occurred.

Illustrative Activities: Establishing care objectives, selecting intended therapy, prescribing as an expression of clinical intent and planning monitoring.

Example Care Contexts: An intended course of care may be established during an inpatient encounter, an ambulatory consultation or a virtual consultation.

Explicit Exclusions: Performing planned care is Clinical Intervention. Ordering, requesting, scheduling and allocating care do not themselves establish that an intervention has occurred. Operational service/resource availability information is not Health Information Access merely because it informs planning.

01.04 Clinical Intervention

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to perform clinical actions intended to alter, maintain or otherwise manage an individual's health state in accordance with an established care intent.

State Change: `Intended care → Care action performed`

Boundary: Clinical Intervention establishes that a care action was performed. An intervention need not produce a successful clinical outcome. Access to an intended intervention does not confer authority to perform it.

Illustrative Activities: Medication administration, surgery, therapy and counselling when performed as clinical actions under an established care intent.

Example Care Contexts: An intervention may be performed in inpatient, community or home care, or through a suitable virtual delivery mode.

Explicit Exclusions: Ordering, requesting, scheduling or allocating an intervention is not itself the Clinical Intervention; selecting the intended response belongs to planning.

01.05 Individual Health Monitoring

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to observe and evaluate an individual's health state over time in order to identify clinically meaningful change, stability or deviation from an expected state or trajectory.

State Change: `Observed health states over time → Change / stability / deviation identified`

Boundary: Monitoring establishes change, stability or deviation. Interpretation of the clinical significance of that change remains Clinical Determination.

Illustrative Activities: Comparing serial observations with a baseline or expected trajectory and identifying a trend or deviation.

Example Care Contexts: Serial inpatient observations and longitudinal home monitoring exercise the same ability across different time patterns and settings.

Explicit Exclusions: Clinical interpretation of an identified change and selection of the care response are not monitoring. Evaluating care effectiveness against its objectives belongs to Individual Care Review.

01.06 Individual Care Review

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to evaluate the progress and effectiveness of an individual's care against its intended objectives and establish the continuing appropriateness of the current care intent.

State Change: `Care progress / outcomes → Effectiveness and continuing appropriateness established`

Boundary: Care Review evaluates the care itself against its intent and objectives; it is not simply another name for reassessment.

Illustrative Activities: Reviewing response to delivered therapy against care objectives and evaluating whether the current care intent remains appropriate.

Example Care Contexts: Review during an admission, a scheduled follow-up or longitudinal care retains the same evaluation purpose.

Explicit Exclusions: Reassessment alone does not establish care effectiveness. Establishing a revised intended course of care belongs to Individual Care Planning.

Clinical Determination and Individual Care Planning have no adjudicated CT3 set in the supplied decisions. Clinical Intervention's Therapy & Counselling Intervention / Medication Intervention / Procedural Intervention candidate remains provisional under DN-BC-010; it is not promoted as a final taxonomy. Examples cannot close these gaps.

The previously approved B2.1 support trace is preserved without extending it to CT3s or establishing runtime dependencies.

CT2 — Individual Health Assessment

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Establish an understanding of an individual's current health state.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Health Assessment; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health History Assessment	Establish relevant health knowledge from the individual's history and applicable historical/contextual information.
Clinical Examination	Establish health findings through clinical examination.
Health Status Measurement	Establish individual health observations or measurements.

Boundary: The decomposition distinguishes means of establishing knowledge. Functional, nutritional, cognitive, mental-health and similar labels are assessment domains, not automatically peer abilities. Risk interpretation belongs to Clinical Determination. Care setting, profession, specialty and information type do not define the decomposition.

CT2 — Individual Health Monitoring

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Observe health over time to identify change, stability or deviation.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Health Monitoring; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health State Monitoring	Establish an aggregate, temporally situated view of the individual's health from applicable monitored data.
Care Response Monitoring	Scope that health-state view to a particular problem/condition and the treatment applied to it.

Boundary: Care Response Monitoring consumes health-state services provided through Health State Monitoring without independently recreating the aggregate view. This semantic consumption establishes no named Service, interface or runtime invocation. Measurement supplies observations; monitoring establishes a view; Clinical Determination interprets its clinical significance. Deterioration, improvement and stability are possible monitored states, not established peer CT3s.

CT2 — Individual Care Review

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Evaluate care against intent and objectives.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Care Review; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Appropriateness Review	Evaluate whether the current intended care remains suitable in the individual's current context.
Care Effectiveness Review	Evaluate whether delivered care is achieving or has achieved its intended goals and outcomes.

Boundary: An effectiveness finding may inform appropriateness without replacing it. Care may be appropriate yet ineffective, or effective yet no longer appropriate to continue. Monitoring the response is distinct from judging effectiveness; identifying a need to reconsider care does not establish the replacement plan. Replanning belongs to Individual Care Planning.

02. Care Access & Coordination

Tier and ancestry: CT1 root.

Definition: Managing patient intake, referral triage, appointment scheduling, multidisciplinary care team coordination, cross-facility patient transfers, and care continuity across organisational boundaries.

Classification: Harmonia-Relevant.

CT2 — Care Access Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Establish and manage access to applicable care.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Access Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Intake Management	Establish and manage the intake context through which an individual seeks or enters care.
Care Referral Management	Manage a request for another participant/service to assess, manage or assume applicable care responsibility.
Care Eligibility Determination	Determine whether applicable eligibility conditions for care access are satisfied.

Boundary: Intake, referral and eligibility are distinct abilities. Referral submission or receipt does not establish acceptance, care delivery or responsibility actually assumed; eligibility does not itself allocate or deliver care.

CT2 — Care Scheduling & Capacity Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Manage scheduled opportunities, care places, usable operational capacity and care-demand resourcing.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Scheduling & Capacity Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Appointment Management	Establish and manage a scheduled opportunity for care.
Care-Place Allocation	Allocate an applicable care place to a care requirement.
Care-Place Readiness Management	Establish and manage whether a particular care place is ready for allocation or use.
Care Resource Deployment	Apply legitimately available and appropriately authorised resources to a current or planned operational care requirement, particularly using roster/on-call and service-coverage context.
Operational Care Flow Coordination	Coordinate operational care flow and its progression within the care-delivery environment.
Operational Care Capacity Management	Understand and manage the service's usable capacity to accommodate care in its current operational context.

Boundary: Available staff, equipment or an unoccupied bed does not establish usable service capacity. Available for service coverage $\neq$ deployed to a care requirement $\neq$ assigned operational work. Care Resource Deployment consumes availability, role, qualification, engagement and privilege context without changing authoritative rosters, qualifications, employment or privilege records. It establishes no Harmonia authority over clinical work, enterprise assets or hospital-wide logistics. Operational capacity is distinct from strategic/tactical capacity planning and from collective service readiness (OI-BC-002).

CT2 — Care Coordination & Transition

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Coordinate care participation, actual responsibility transition and discharge.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Coordination & Transition; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care-Team Coordination	Coordinate participating care-team context and collaboration around care.
Care Responsibility Transition	Establish and coordinate who has actually assumed applicable care responsibility.
Care Discharge Coordination	Coordinate applicable discharge requirements and progression.

Boundary: Patient moved ≠ care responsibility transferred. Arrival, referral submission, message receipt, discharge readiness or physical departure does not itself establish acceptance of responsibility. Information assembly for clinical handover does not confer clinical-handover management authority on Harmonia.

CT2 — Care Support Logistics Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Coordinate bounded operational support needed for care access and fulfilment.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Support Logistics Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Patient Movement Coordination	Coordinate physical patient transit and applicable mobility constraints.
Clinical Material Logistics Coordination	Coordinate clinically relevant material movement, custody, availability or disposition needed for care or diagnostic fulfilment.
Care Support Work Coordination	Coordinate authorised operational support work: what needs doing, who performs it and how progression, completion or escalation is coordinated.

Boundary: These abilities do not establish a general hospital logistics, supply-chain, ERP, facilities or workforce-management architecture. Support work coordination does not determine clinical work; assignment/dispatch does not establish availability, qualification, privilege or clinical authority.

03. Health Rights, Advocacy & Participation

Tier and ancestry: CT1 root.

Definition: Protecting patient rights, facilitating informed consent, managing advance care directives, supporting patient advocacy, and enabling consumer participation in health decisions.

Classification: Adjacent.

No CT2/CT3 set is established by this reconciliation. Consent and record-access evidence can support rights without assigning advocacy or participation management to Harmonia.

04. Diagnostic, Therapeutic & Clinical Support Services

Tier and ancestry: CT1 root.

Definition: The specialised clinical services that support direct patient care, including pathology and laboratory testing, diagnostic imaging (radiology), pharmacy and medication dispensing, and allied health therapies.

Classification: Harmonia-Relevant.

The following diagnostic and pharmacy branches are established. Other therapeutic/support scope remains enterprise context; no further subordinate identity is manufactured.

CT2 — Diagnostic Support Provision

Tier and ancestry: CT2; parent Diagnostic, Therapeutic & Clinical Support Services (CT1).

Definition: Provide diagnostic services and manage their evidence, interpretation and governed reporting.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Diagnostic Support Provision; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Diagnostic Service Appointment Management	Manage the diagnostic-service-specific scheduled opportunity, including preparation, modality, urgency, duration and other service-specific constraints.
Diagnostic Service Delivery	Perform the diagnostic activity that obtains or produces diagnostic evidence.
Diagnostic Specimen Management	Maintain a specimen's identity, integrity, suitability, custody and applicable lifecycle context.
Diagnostic Data Management	Manage acquired or produced diagnostic evidence as information, including identity, association, integrity, organisation, availability and lifecycle.
Diagnostic Interpretation	Establish what diagnostic evidence demonstrates within the diagnostic service's professional scope.
Diagnostic Report Management	Establish and govern the authoritative expression of diagnostic findings, including applicable creation, review, amendment, verification, finalisation and authorisation/signing.
Diagnostic Report Distribution	Provide an established report to applicable authorised recipients or consuming contexts under applicable access, privacy, security and communication controls.

Boundary: Scheduling $\neq$ performance $\neq$ evidence management $\neq$ interpretation $\neq$ governed report $\neq$ distribution. Specimen and data responsibilities remain distinct. Diagnostic interpretation does not establish patient-level Clinical Determination from the total clinical context. Generic appointments are distinct from diagnostic-specific constraints. Diagnostic Order Management is not established here as a subordinate capability. Signing/finalisation are report behaviours, not additional CT3s. Services may exercise a subset, concurrently or iteratively; no mandatory diagnostic workflow is established.

CT2 — Medication & Pharmacy Services

Tier and ancestry: CT2; parent Diagnostic, Therapeutic & Clinical Support Services (CT1).

Definition: Provide professional medication/pharmacy services under applicable controls.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Medication & Pharmacy Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Prescription Medication Dispensing	Professionally prepare and provide medication according to an applicable authorised prescription/order and dispensing requirements.
Non-Prescription Medication Provision	Provide medication not requiring an applicable prescription, subject to professional, regulatory and safety controls.
Medication Advice	Provide professional medication-specific advice about appropriate use, precautions, interactions, administration, adherence or safe handling.
Medication Dose Administration Aid Provision	Prepare and provide organised medication doses supporting administration according to an already established regimen.

Boundary: Medication prescribed $\neq$ dispensed $\neq$ provided $\neq$ administered. Advice does not establish diagnosis, prescribing or authority to alter treatment. Dose aid provision does not establish the regimen; commercial packaging examples do not define the capability. These are pharmacy responsibilities, distinct from clinical intervention, and do not prescribe a medication sequence. Medication Advice's detailed scope remains subject to refinement. OI-BC-001

remains unresolved; compounding/preparation is neither established, rejected nor silently subsumed.

05. Health Products & Clinical Technology

Tier and ancestry: CT1 root.

Definition: Manage health products and clinical technology in the care environment, with distinct device inventory, functional/release, deployment, utilisation, integration and lifecycle responsibilities. Product procurement and supply context is retained without inferring further Health Product Management decomposition.

Classification: Harmonia-Relevant.

CT2 — Clinical Technology Management

Tier and ancestry: CT2; parent Health Products & Clinical Technology (CT1).

Definition: Manage clinical technology as a coherent business concern rather than only the device information Harmonia happens to use.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Technology Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Device Inventory Management	Establish what devices exist and their managed asset identity, specification, ownership/custody and inventory context.
Clinical Device Feature & Release Management	Manage functional/release state, including applicable software, firmware, configuration and enabled device characteristics.
Clinical Device Loan & Deployment Management	Manage device loan, issue, assignment, placement, return and deployment.
Clinical Device Utilisation Support	Enable and support otherwise appropriately authorised users to use the particular device safely and effectively.
Clinical Device Integration Provision	Establish and maintain the device's capability and configuration to participate in applicable information exchanges.
Clinical Device Data Integration	Acquire, exchange or convey device data through established integration capability.
Clinical Device Lifecycle Management	Govern introduction, continued service, review, replacement, withdrawal, decommissioning and retirement within the overall device estate.

Boundary: Inventory $\neq$ release state $\neq$ deployment $\neq$ user enablement $\neq$ use in care. Professional qualification/privilege does not establish device-specific enablement. Integration provisioned $\neq$ data integrated; neither confers clinical interpretation or downstream information authority. Device monitoring remains externally provided enablement, with provider unestablished. Monitoring, maintenance and lifecycle disposition have distinct meanings. The Maintenance membership discrepancy is preserved under Unresolved Business Capability matters; no eighth CT3 is inferred.

06. Clinical Quality, Safety & Improvement

Tier and ancestry: CT1 root.

Definition: Operational clinical quality, safety and improvement: monitoring clinical outcomes; identifying quality and safety concerns; adverse event and incident management, reporting and response; infection control; operational quality and safety monitoring; continuous quality improvement; and coordination of operational responses to identified concerns.

Classification: Harmonia-Relevant.

CT2 — Clinical Safety Management

Tier and ancestry: CT2; parent Clinical Quality, Safety & Improvement (CT1).

Definition: Manage the clinical-risk distinctions materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Safety Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Risk Identification	Recognise and characterise a clinical safety risk from applicable evidence.
Clinical Risk Mitigation Management	Manage prospective/preventative measures reducing identified risk likelihood, exposure or consequence.
Clinical Risk Response Management	Manage active response, containment and remediation where risk has materialised, is emerging or requires active intervention.

Boundary: Monitoring supplies evidence but does not itself identify/characterise clinical risk. Monitoring providers remain unestablished. Mitigation/response may require action by other capabilities without acquiring those capabilities' ownership. Response is not a generic enterprise incident-management container. The three CT3s are deliberately non-exhaustive specialist context; identified, mitigated and actively responded-to states are distinct, not a mandatory sequence.

CT2 — Clinical Quality Management

Tier and ancestry: CT2; parent Clinical Quality, Safety & Improvement (CT1).

Definition: Manage the clinical-quality distinctions materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Quality Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Quality Criteria Management	Establish and manage measures, indicators, expectations and criteria against which clinical quality is considered.
Clinical Quality Evidence Management	Manage the body of applicable evidence used for clinical-quality consideration without owning its originating activity.
Clinical Quality Review & Assessment Management	Review evidence against applicable criteria and establish a clinical-quality assessment, finding or conclusion.
Clinical Quality Reporting	Communicate established clinical-quality measures, assessments, findings and conclusions to authorised consumers.

Boundary: Criteria $\neq$ evidence $\neq$ assessment $\neq$ reporting $\neq$ subsequent improvement. Observation alone is not quality judgement and reporting does not originate the reported conclusion. Clinical Quality Performance Monitoring is not established as a separate CT3; the provider of monitoring evidence remains unestablished. Continuous Improvement is not a subordinate CT3 here; generic placement remains unestablished. Health Service Continuous Improvement has the separately established service-wide purpose under 14. The four CT3s do not exhaust specialist Clinical Quality.

07. Community Health & Wellbeing

Tier and ancestry: CT1 root.

Definition: Support community health and wellbeing, including the bounded preventative-health abilities required for R1/R2. Wider environmental health, outreach and social determinants remain enterprise context without inferred additional decomposition.

Classification: Harmonia-Relevant.

CT2 — Preventative Health

Tier and ancestry: CT2; parent Community Health & Wellbeing (CT1).

Definition: Address preventative-health needs through organised activity with sufficient R1/R2 distinctions.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Preventative Health; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Preventative Health Problem Identification	Identify and characterise a preventative-health problem warranting organised attention.
Preventative Health Programme Management	Establish and manage an organised response to a preventative-health need/objective without absorbing its delivery capabilities.
Preventative Health Education Services	Provide education intended to prevent adverse health outcomes and influence preventative knowledge or behaviour.
Preventative Health Programme Monitoring	Observe programme operation, reach, participation, coverage, progress and applicable performance.
Preventative Health Reporting	Report established preventative-health problems, programmes, cohorts, activity, performance and outcomes.
Preventative Health Cohort Management	Establish and manage the target population, inclusion/exclusion criteria, membership, stratification and applicable entry/exit context.

Boundary: Screening and immunisation are programme types, not automatically separate CT3s. Population at risk $\neq$ defined cohort $\neq$ individual clinical problem. Programme management does not own clinical intervention, appointments or other delivery abilities. Education is purpose-specific rather than generic communication. Programme monitoring is distinct from population-health assessment, individual monitoring and assurance/review conclusions. This is deliberate specialist under-decomposition; unmodelled responsibilities are not assigned to existing CT3s.

08. Population Health & Health-System Planning

Tier and ancestry: CT1 root.

Definition: Understand population health through assessment/surveillance and applicable event/notification management; assess health-service demand and plan required capacity and its distribution.

Classification: Harmonia-Relevant.

CT2 — Population Health Surveillance

Tier and ancestry: CT2; parent Population Health & Health-System Planning (CT1).

Definition: Understand population-health state and manage organised surveillance, significant events and applicable notification.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Population Health Surveillance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Population Health Longitudinal Assessment	Establish population-health state, patterns and change over time from applicable evidence.
Population Health Surveillance Management	Establish and manage surveillance purposes, populations, concerns and regimes.
Population Health Event Identification	Identify and characterise significant population-health events/patterns from applicable evidence.

CT3 Business Capability	Definition
Notifiable Health Event Management	Manage formal notification applicability, required information, recipient, status, submission, correction/supplementation and applicable follow-up.
Population Health Reporting	Communicate established population-health assessments, surveillance, events, trends and findings.

Boundary: Population state ≠ individual state; observed change ≠ identified event ≠ notifiability ≠ obligation discharged. Notification management neither defines the obligation nor owns operational response, and is distinct from general reporting. Surveillance may provide evidence for preventative problems/cohorts, demand planning, clinical quality or safety without establishing those outcomes. Population assessment differs from preventative-programme performance. Individual observations may contribute population evidence without becoming population conclusions. Epidemiology, outbreak response and public-health scope are deliberately not exhaustively modelled.

CT2 — Health-System Planning

Tier and ancestry: CT2; parent Population Health & Health-System Planning (CT1).

Definition: Assess health-service demand and plan the capacity and footprint required to meet it.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health-System Planning; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Demand Assessment	Assess current/expected health-service demand across a population, geography, service domain or health-system context.
Health Service Capacity Planning	Determine required strategic/tactical capacity to address assessed demand.
Health Service Distribution Planning	Determine the distribution of planned capacity across a geographical, organisational, site or service-network footprint.

Boundary: Population-health evidence does not predetermine a demand conclusion. Individual care demand differs from health-system demand; required capacity differs from established or operationally usable capacity. Planning owns neither underlying resources nor operational allocation/deployment. Planned footprint differs from managed locations and identifiable offerings. Detailed commissioning, funding, capital, infrastructure and workforce planning remain intentionally unmodelled.

Region 2: Health Information & Digital Health

09. Health Information & Knowledge Management

Tier and ancestry: CT1 root.

Definition: Manage governed health-information content, structures, lifecycle, catalogues, compartments, correlation and retrieval, alongside reusable clinical knowledge and its governed standing.

Classification: Harmonia-Core.

CT2 — Health Information Management

Tier and ancestry: CT2; parent Health Information & Knowledge Management (CT1).

Definition: Manage health information as governed, semantically meaningful content.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Information Data Management	Manage actual health-information content within applicable governed structures.
Health Information Structure Management	Manage structures, element/reference definitions, constraints and semantic bindings giving content unambiguous meaning.
Health Information Lifecycle Management	Govern state, amendment, correction, supersession, currency, retention and disposition while preserving provenance and historical meaning.
Health Information Catalogue and Index Management	Manage descriptive catalogues/indexes through which information existence and location can be established.
Health Information Compartmentalisation	Manage contextual association, partitioning and bounded information groupings.
Health Information Data Correlation	Establish meaningful relationships between independently managed information items/sets.
Health Information Data Retrieval	Obtain identified information under an applicable authorised request/context.

Boundary: Semantic authority ≠ semantic binding ≠ content; a persisted value is not self-describing health information. Structure/binding management does not govern the referenced terminology/value set. Catalogue existence/location ≠ retrieval ≠ communication ≠ presentation; discoverability does not authorise access. Compartment membership may inform but does not determine authorisation. Correlation establishes neither clinical interpretation nor entity identity. Current state and immutable historical operational truth remain distinct; managing information does not originate its facts. Structures are not equated with FHIR resources, physical schemas or storage mechanisms.

CT2 — Clinical Knowledge Management

Tier and ancestry: CT2; parent Health Information & Knowledge Management (CT1).

Definition: Identify, extract, formulate, associate, govern and share reusable clinical knowledge.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Knowledge Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Knowledge Context Identification	Identify sources/contexts potentially containing reusable clinical knowledge.
Clinical Knowledge Context Extraction	Extract relevant material while preserving its source/context relationship.
Clinical Knowledge Information Formulation	Formulate explicit reusable knowledge, including applicability, qualifications, rationale and evidence relationships.
Clinical Knowledge Concept Association	Associate knowledge with applicable governed clinical concepts without acquiring concept authority.
Clinical Knowledge Review & Approval Management	Manage review, endorsement, approval/rejection, conditions and applicable history for intended governed use.
Clinical Knowledge Publication & Sharing	Make approved knowledge available to applicable authorised consumers and use contexts.

Boundary: Source identified ≠ extracted material ≠ formulated knowledge ≠ reviewed/approved knowledge ≠ publication ≠ individual application. Reusable knowledge differs from source health information and individual clinical facts. Formulation is distinct from Health Information Structure Management and does not establish approval. Concept association does not redefine terminology or semantic authority. Publication establishes neither individual applicability nor a clinical conclusion/decision. Clinical knowledge and source provenance/authority

remain distinguishable; no mandatory knowledge workflow is prescribed.

10. Standards, Semantics & Reference Governance

Tier and ancestry: CT1 root.

Definition: Govern organisational adoption, oversight and appropriate use of health-information models, standards, terminology, classifications and semantic mappings.

Classification: Harmonia-Core.

CT2 — Health Information Standards Governance

Tier and ancestry: CT2; parent Standards, Semantics & Reference Governance (CT1).

Definition: Govern organisational adoption and use of health-information models and applicable standards.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Standards Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Information Model Discovery	Identify information models relevant to the organisation's health-information needs.
Information Model Adoption	Adopt applicable information models, including intrinsic assessment of adoption impacts.
Information Model Education	Support organisational understanding and appropriate use of adopted information models.
Information Model Compliance	Address organisational conformity to applicable information-model requirements.
Information Model Governance Oversight	Oversee and evaluate effectiveness, consistency and appropriate operation of information-model governance.
Standards Adoption	Adopt applicable standards within the organisation.
Standards Impact Identification	Identify standards impacts across clinical/work practice, permitted medications, procedures, policy, safety, obligations and reporting as applicable.

Boundary: Oversight does not perform its discovery, adoption, education or compliance peers. Model-adoption impact is intrinsic to adoption; standards impacts can extend beyond information-system implementation and warrant their distinct established identity. These are the peers explicitly named in DN-BC-025; no additional standards peer is inferred to fill out a conventional taxonomy. Conceptual progression does not establish a workflow.

CT2 — Clinical Semantics Governance

Tier and ancestry: CT2; parent Standards, Semantics & Reference Governance (CT1).

Definition: Govern clinical meaning and appropriate organisational use of semantic artefacts.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Semantics Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Terminology Governance	Manage governed terminology/reference-set lifecycle and standing, including acceptance, release, update, supersession/retirement and authority/provider coordination.
Terminology Utilisation Oversight	Oversee actual terminology/reference-set use and identify inappropriate, inconsistent or drifting usage.

CT3 Business Capability	Definition
Terminology Localisation	Manage controlled local responses where adopted terminology does not meet legitimate organisational semantic requirements.
Classification Governance	Govern applicable classification definitions and their organisational standing.
Classification Utilisation Oversight	Oversee appropriate organisational application of classifications.
Semantic Mapping Governance	Govern mappings between applicable semantic artefacts without collapsing their source meanings or authorities.

Boundary: Compartmentalisation concerns business/context relationships; classification concerns sensitivity and consequences of exposure/mishandling; terminology concerns consistent meaning. They are orthogonal even where metadata/mechanisms are shared. Classification Localisation is not established and SHALL NOT be invented for symmetry. Governance differs from structure binding and knowledge-to-concept association.

11. Connected Health Services

Tier and ancestry: CT1 root.

Definition: Exchange health information appropriately and provide digitally mediated communication, notifications and collaborative interaction between health-service participants.

Classification: Harmonia-Core.

CT2 — Health Information Exchange

Tier and ancestry: CT2; parent Connected Health Services (CT1).

Definition: Exchange health information appropriately between participating parties, systems and services.

No CT3 decomposition is established here.

Boundary: This organisational ability is not a FHIR server, interface engine, protocol or interoperability product. The established rename from Health Information Connectivity does not allocate additional CT3s. Health Information Sharing Programme Management is a named working capability in DN-BC-027 whose precise tier/parent is not supplied there; it is retained separately as unresolved. The collision with the existing BEC name remains for explicit adjudication; no BEC identity is merged or renamed.

CT2 — Digital Communication Channel

Tier and ancestry: CT2; parent Connected Health Services (CT1).

Definition: Provide digitally mediated communication, notification and collaborative interaction between patients, practitioners and other health-service participants.

No CT3 decomposition is established here.

Boundary: Appointment notifications, pharmacy queries, results and collaboration illustrate participant communication. This is not the general UI/presentation capability: exercising another capability through a UI does not itself exercise this communication ability. General presentation enablement remains parked for BEC reconciliation. No CT3 set is established by DN-BC-027.

12. Security, Privacy & Digital Trust

Tier and ancestry: CT1 root.

Definition: Establish adequate identity confidence, coherent access controls, privacy/protection safeguards, governed activity attribution and accountability for security-relevant activity and information use.

Classification: Harmonia-Core.

CT2 — Digital Identity & Access Governance

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Govern sufficient identity confidence and coherent access controls across the organisation.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Digital Identity & Access Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Identity Assurance	Establish sufficient confidence that a person is who they claim to be through appropriate evidence and processes.
Identity Modelling & Interoperability	Establish identity models and interoperability arrangements appropriate to organisational identity use.
Identity Qualification Control	Determine whether presented identity characteristics, provenance and assurance quality are sufficient for its intended context.
Identity Identification & Authentication	Identify an actor and authenticate the asserted identity.
Access Control Context Management	Determine what information, functions, activities or other concerns require control, under what circumstances and how requirements are modelled.
Access Control Assurance & Reporting	Establish organisational assurance/evidence of whether access-control arrangements are appropriately established and operating.
Access Control Collaboration	Coordinate arrangements across independently controlled domains/parties while retaining their control ownership.
Access Control Enforcement	Ensure physical, procedural and digital control mechanisms coherently impose intended controls.

Boundary: Who are you? $\neq$ confidence in the claimed identity $\neq$ identity quality sufficient here. Federation and Single Sign-On are examples, not equivalent concepts or universal identity-quality guarantees. Context management is not merely runtime attribute assembly; enforcement is not a software authorisation engine. Strong local control does not imply end-to-end control. Identity Assurance is an organisational ability; the possible future BEC name Identity Assurance Representation remains unestablished.

CT2 — Health Information Protection

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Govern privacy consequences and adequate protection of the information estate and its environment.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Protection; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Privacy Safeguard Management	Establish organisational safeguards and assurance processes concerning privacy consequences of information supply, exposure or use.

CT3 Business Capability	Definition
Health Information Safeguarding	Ensure information and its operational environment are protected and resilient against loss, compromise, inappropriate access or operational failure.
Health Information Access Monitoring	Evaluate actual organisational access/handling practice, including controls, accessibility, compartments, classification and physical/printed handling.
Health Information Compartment Assurance	Assure appropriate establishment and application of compartmentalisation without establishing access decisions.
Health Information Classification Assurance	Assure appropriate classification according to sensitivity and potential consequences of exposure/mishandling.

Boundary: Privacy consequences differ from estate/environment protection. Storage, backups, disaster recovery, physical controls and testing illustrate organisational safeguards rather than define software components. Access Monitoring evaluates practice using IT, audit, physical-access or other evidence; it is not merely observation of software access events. Evidence supply does not transfer business accountability. These subject-specific protection abilities are not separate macro-level Health Service Assurance trees.

CT2 — Digital Accountability & Trust

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Establish governed attribution and accountability for activity and information use.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Digital Accountability & Trust; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Governed Activity Attribution	Preserve who participated, what was done, the attributable person/role/unit/organisation capacity and applicable permission/authority/constraint context.
Security & Information-Use Accountability	Evaluate whether attributed security-relevant activity/information use was appropriate within policy, obligation and authority constraints, and establish accountability.

Boundary: Actor identified $\neq$ activity attributed $\neq$ activity permitted. Attribution without permission/constraint context cannot establish entitlement and is richer than simple audit logging or provenance. Accountability evaluates appropriateness rather than merely recording participation. Expected consumption by governance/assurance or other enablement establishes no structural BC$\leftrightarrow$BEC relationship.

Region 3: Research & Innovation

13. Health Research & Innovation

Tier and ancestry: CT1 root.

Definition: Provide existing health information for authorised research under applicable constraints and evaluate health innovation; deliberately under-decomposed for R1/R2.

Classification: Harmonia-Relevant.

CT2 — Research Information Support

Tier and ancestry: CT2; parent Health Research & Innovation (CT1).

Definition: Provide the existing health information needed for authorised research within the deliberately bounded R1/R2 responsibility.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Research Information Support; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Governed Research Information Provision	Provide existing health information for an authorised research purpose under applicable ethics, governance, privacy, consent, security and information-use constraints.

Boundary: This is deliberate under-decomposition, not a claim that information provision exhausts research support. Research Assessment and Governance, Research/Trials Oversight, clinical-trial management, genomic analysis and other specialist research abilities are intentionally unmodelled unless Harmonia requirements make them material. Their absence does not assign them to information provision.

CT2 — Health Innovation

Tier and ancestry: CT2; parent Health Research & Innovation (CT1).

Definition: Consider health-service and clinical innovation within the R1/R2 business context.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Innovation; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Innovation Evaluation	Evaluate proposed/emerging innovation for suitability, value, implications and potential adoption.

Boundary: Evaluation does not establish adoption or the clinical/operational authority to implement innovation. The single CT3 is deliberately sufficient, not an exhaustive innovation lifecycle.

Region 4: Health Service Management

14. Health Service Direction & Stewardship

Tier and ancestry: CT1 root.

Definition: Model needed health services and their viability requirements, support establishment/improvement and align services with the wider ecosystem of dependencies, resources, financial arrangements and organisational change.

Classification: Harmonia-Relevant.

CT2 — Health Service Modelling

Tier and ancestry: CT2; parent Health Service Direction & Stewardship (CT1).

Definition: Establish what service should exist and what it needs to become viable.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Modelling; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Need Identification	Establish an unmet health-service need requiring response.
Health Service Capability Identification	Identify the ordinary business abilities needed to satisfy the service need across clinical, workforce, facility, technology, information and support concerns.
Health Service Implementation Support	Support establishment of the framed service through applicable organisational preparation and programmes of work.

CT3 Business Capability	Definition
Health Service Continuous Improvement	Evaluate the established service and identify correction/ improvement needs that may feed back into modelling and implementation support.

Boundary: Capability Identification uses ordinary business meaning; it is not Harmonia CT-tier allocation. Implementation support may require recruitment, training, facility, equipment or technology change without absorbing HR, facilities, ICT or procurement responsibility. Identification, fulfilment requirements, support and review describe semantic relationships, not a prescribed workflow.

CT2 — Health Service Alignment

Tier and ancestry: CT2; parent Health Service Direction & Stewardship (CT1).

Definition: Position the service coherently in its wider health-service ecosystem.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Alignment; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Co-Dependency Management	Identify and manage what the service needs from others and what others need from it.
Health Service Resource Identification	Identify required workforce, facilities, equipment, clinical technology, ICT, administration and support resources without owning them.
Health Service Financial Management	Establish the service's fit within funding, financial and charging arrangements without absorbing enterprise finance/accounting.
Health Service Implementation Planning	Establish and coordinate surrounding organisational/service changes needed to introduce or materially change the service.

Boundary: Alignment does not establish the authoritative service catalogue; it positions the service within the environment represented by that catalogue. Modelling determines what service is needed; alignment determines how it fits; Offering Management establishes the identifiable offering. Expected future Service Catalogue richness explains architectural relevance, but establishes neither a BEC identity/hierarchy nor a BC↔BEC mapping. Alignment does not resolve operational-readiness ancestry.

15. Workforce & Organisational Capability

Tier and ancestry: CT1 root.

Definition: Govern practitioner qualification, engagement/privilege and enterprise service coverage, together with organisation identity/structure, recognised locations and identifiable healthcare service offerings.

Classification: Harmonia-Relevant.

Workforce & Organisational Capability remains the established CT1. Health Workforce Management and Healthcare Organisation & Service Structure Management are its distinct CT2 frames; directories represent their facts without replacing business governance of those facts.

CT2 — Health Workforce Management

Tier and ancestry: CT2; parent Workforce & Organisational Capability (CT1).

Definition: Govern practitioners' professional standing, engagement/privilege and whole-enterprise service coverage.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Workforce Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Practitioner Qualification Management	Govern professional/clinical standing and qualifications, including applicable review, escalation, complaints, penalties/issues, suspension and revocation.
Practitioner Engagement & Privilege Management	Manage practitioners as service participants, their roles/ capacities, applicable privileges and qualification context above ordinary HR employment administration.
Workforce Service Coverage Management	Observe whether required enterprise work/service obligations can be fulfilled, identify coverage gaps/risks and adjust coverage in response.

Boundary: Qualified $\neq$ engaged $\neq$ privileged for a particular activity. Coverage is broader than rostering; it asks whether required work can be covered and how coverage should change. Care Resource Deployment applies available resources to a particular care requirement; Care Support Work Coordination progresses particular support work. Neither consumption of qualification/coverage information nor directory representation transfers the underlying workforce authority to Harmonia.

CT2 — Healthcare Organisation & Service Structure Management

Tier and ancestry: CT2; parent Workforce & Organisational Capability (CT1).

Definition: Manage organisational identity/structure, recognised locations and identifiable organisational service offerings.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Healthcare Organisation & Service Structure Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Healthcare Organisation Identity & Structure Management	Establish and manage identity and structural relationships of participating organisations and organisational units.
Healthcare Location Definition Management	Establish and manage recognised locations and their applicable business characteristics and relationships.
Healthcare Service Offering Management	Establish/manage what identifiable service is offered, by whom, where, when and under which availability conditions/constraints.

Boundary: Location definition is distinct from Facilities Management. Service exists $\neq$ offered here $\neq$ available at this time/context. Temporal/conditional availability is material: a walk-in offering may operate 06:00–22:00 while another pathway covers other hours. Offering availability does not establish collective operational readiness; OI-BC-002 remains unplaced. Organisation, location and practitioner associations do not substitute for material Healthcare Service context.

16. Corporate Resources & Enterprise Services

Tier and ancestry: CT1 root.

Definition: Provide the bounded clinical-activity charging information and enterprise support context required for R1/R2, without assigning enterprise finance, HR, facilities or ICT operating authority to Harmonia.

Classification: Harmonia-Relevant.

CT2 — Financial & Commercial Services

Tier and ancestry: CT2; parent Corporate Resources & Enterprise Services (CT1).

Definition: Provide the financial/commercial business context materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Financial & Commercial Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Activity Charging Information Provision	Identify and provide information about chargeable or cost-relevant clinical/health-service activity where required for applicable funding, financial or charging purposes.

Boundary: The single CT3 is deliberately narrow. Referral forwarding, diagnostic requests, service types, quantities and utilisation context can supply evidence; not every mediated activity is chargeable. Evidence provision does not confer pricing, billing, claims, accounting or revenue-management responsibility. Comprehensive Medicare, insurance, activity-based funding and revenue-cycle architecture is not established.

CT2 — Corporate Support Services

Tier and ancestry: CT2; parent Corporate Resources & Enterprise Services (CT1).

Definition: Provide enterprise support whose resources, availability, state or constraints may materially affect service activity.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Corporate Support Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Back-Office Services	Provide administrative/support services on which health-service activity depends.
Facilities Management	Manage physical facilities/services and applicable resource, location, availability and facility-state context.
IT Infrastructure Management	Operate/manage underlying technology infrastructure and its availability.
Communications & Publicity	Manage organisational communication/publication arrangements relevant to information availability/distribution.
People Management	Manage people across the organisation, distinct from practitioner-specific Health Workforce Management.
Operational Oversight	Observe broader operational conditions affecting the organisation’s ability to work or deliver services.

Boundary: These are relevant enterprise abilities even where Harmonia only consumes their state/resource/constraint context; contextual consumption alone does not make them Harmonia-Core. They are not decomposed further for R1/R2 absent a material requirement. Consumption transfers neither resource ownership nor corporate operating authority and does not resolve OI-BC-002.

Region 5: Governance & Assurance

17. Health Service Governance

Tier and ancestry: CT1 root.

Definition: Establish and maintain the obligations, authorities, constraints, policies and governance framework under which health-service activity must operate, and define how adherence will be assessed, checked and reported.

Classification: Unresolved.

Health Service Governance establishes what the organisation must do and how adherence will be determined and governed. It does not determine operational means or acquire progression/management of governed work. Enabling derivation remains unestablished; consumption of its requirements alone does not settle its relevance classification.

CT2 — Governance Framework Management

Tier and ancestry: CT2; parent Health Service Governance (CT1).

Definition: Establish the authoritative WHAT and legitimate working parameters.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Governance Framework Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Policy / Obligation Governance	Identify, interpret, establish and maintain authoritative legislative, regulatory, policy, standards, guideline, contractual and other obligations, attributable and traceable to source authority.
Authority & Constraint Governance	Establish who may make, approve or govern organisational decisions/artefacts and the bounds within which authority may be exercised.

Boundary: Obligation = what must be satisfied; authority = who may decide/approve; constraint = within what bounds. Engagement with regulators, standards/compliance bodies or other authorities can establish legitimate parameters without transferring their authority.

CT2 — Compliance Governance

Tier and ancestry: CT2; parent Health Service Governance (CT1).

Definition: Define how adherence to the authoritative WHAT will be determined, checked and reported.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Compliance Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Compliance Assessment Definition	Define the evidence, measures, criteria, controls and assessment arrangements sufficient to evaluate compliance with established obligations, authorities and constraints.
Governance Reporting & Checkpoint Definition	Establish where/how adherence is checked and what governance/compliance information must be reported.

Boundary: Obligation definition establishes what must be satisfied; assessment definition establishes the basis for determining satisfaction. This replaces the candidate Compliance Obligation Definition wording. Compliance Governance does not itself perform assurance or assume operational management.

18. Health Service Assurance

Tier and ancestry: CT1 root.

Definition: Establish and maintain justified confidence that health services can be, and are being, delivered in accordance with applicable expectations, obligations, constraints and required levels of performance.

Classification: Harmonia-Relevant.

Health Service Assurance establishes whether sufficient evidence exists to give justified confidence in the organisation's ability to deliver its health services appropriately. Its macro-level subject is that ability. Security, information, technology, process, clinical and other concerns may contribute criteria/evidence without becoming separate Business Assurance Capability trees. The independently governed evaluation already established in the canonical architecture is a required assurance approach where REQ-FND-005 applies; the reconciliation below preserves its requirements.

CT2 — Health Service Assurance Specification

Tier and ancestry: CT2; parent Health Service Assurance (CT1).

Definition: Specify how macro-level service-delivery assurance will be undertaken and what sufficiently supports confidence.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Assurance Specification; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Assurance Approach Definition	Define scope, evidence expectations, responsibilities, methods, applicable degree of independence and timing/frequency for service-delivery assurance.
Assurance Evaluation Criteria Governance	Establish and maintain criteria against which sufficient assurance can be determined.

Boundary: Criteria may draw on workforce, facilities, clinical practice, information, technology, security, privacy, governance, dependencies and operational state. These contributing concerns do not create separate Business Assurance trees. An assurance approach SHALL preserve every applicable higher-level independence requirement; it cannot designate self-attestation as sufficient where REQ-FND-005 requires independent governed assurance.

CT2 — Assurance Evaluation

Tier and ancestry: CT2; parent Health Service Assurance (CT1).

Definition: Determine what justified confidence the applicable evidence supports.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Assurance Evaluation; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Assurance Evidence Assessment	Evaluate available evidence against the established assurance approach and criteria.
Assurance Conclusion Determination	Establish the legitimately supported conclusion, including insufficiency, qualification and unresolved concerns where applicable.

Boundary: Evidence assessment $\neq$ conclusion determination $\neq$ operational response. Insufficient evidence establishes neither assurance nor non-assurance. Assurance manages its own evaluation, not its subject. These BC responsibilities do not remap existing Assurance Design, Assurance Criteria Management or Governed Assurance BEC ownership.

Assurance Independence Reconciliation

Independence is a required property of applicable assurance approaches/evaluations, rather than the exhaustive definition of the macro-level Business Capability. This does **not** make Harmonia's already-required independent governed assurance optional. REQ-FND-005 requires it for activities, information and outcomes **for which assurance is required**. Its applicability is bounded; it neither defines every enterprise assurance approach nor requires assurance of every activity.

The approved Strategy derivation and Business assurance responsibilities implement that obligation through independently governed progression and conclusion authority. Assurance Approach Definition may determine applicable methods, responsibilities and degree of independence but SHALL NOT weaken those requirements. The subject may contribute evidence; it shall not solely determine, suppress, manufacture or retrospectively alter its assurance outcome. Evidence dependency is distinct from control dependency. No external assessor or physical separation is implied.

The broader organisational capability concerns justified confidence in service delivery. Harmonia's assigned participation remains assurance of explicitly required facts/behaviour; it does not acquire clinical-adequacy assurance or management of the service. Thus the old independent-evaluation definition is preserved as the established governed-assurance obligation within the broader capability, rather than discarded or universalised to all enterprise evaluations. Operational uncertainty under AX-15/REQ-FND-004 remains distinct from evidence insufficiency under REQ-FND-005.

Clinical Services Delivery Assurance Boundary

Health Service Assurance SHALL NOT be represented as Clinical Services Delivery Assurance. The enterprise scope's reference to independent clinical audit/evaluation does not make Harmonia responsible for assuring clinical judgement, professional practice, clinical adequacy of care, or delivery of clinical services by healthcare practitioners or healthcare organisations.

Information concerning clinical service delivery MAY be the subject of governed assurance where an explicit applicable requirement establishes that assurance concern. This does not transfer responsibility for Clinical Services Delivery Assurance to Harmonia. Capability 06 — Clinical Quality, Safety & Improvement retains its established clinical quality/safety responsibilities.

> **Harmonia may assure facts and behaviour concerning clinical activity where explicitly required; it does not thereby assure the clinical adequacy of care.**

Meaning of Independent

Independent Assurance requires an independently governed assurance activity/workflow whose progression and assurance conclusion are not controlled by the activity or mechanism responsible for performing or managing the subject being assured.

Independence does not inherently require an external auditor, a different organisation, department, human actor, application, deployment, database or Kubernetes cluster, or physical infrastructure separation. This is a responsibility and authority boundary consistent with REQ-FND-005, not an Application or Technology Architecture decision about physical execution separation.

> **Evidence dependency does not compromise assurance independence; control dependency does.**

The subject being assured may supply evidence. It shall not thereby be solely responsible for determining, suppressing, manufacturing or retrospectively altering its assurance conclusion.

Assurance / Operational Management Boundary

Health Service Assurance does not thereby assume operational management of its subject, assignment, delegation, reassignment, operational escalation, operational remediation, workflow progression, Incident Management or Incident Response. Findings, exceptions and conclusions may inform or initiate appropriate operational activity; responsibility for that activity remains with its operational capability.

> **Assurance is the QA service, not the Job Foreman.**

An assurance activity may manage its own internal assurance workflow without thereby assuming management of the subject being assured.

Governance, Management, Monitoring and Assurance Boundaries

Concern	Purpose and responsibility	Boundary
Governance	Establishes what must be done or must be true; applicable obligations, constraints and authority; reporting required to demonstrate compliance; and checkpoints required to determine whether obligations and constraints are satisfied.	Establishes the normative operating envelope without determining operational means.
Management	Establishes and coordinates what must be done operationally and how it will be performed within constraints; operational-state monitoring, activity progression, progress reporting, assignment/ reassignment, escalation necessary for completion, response and remediation.	Owns progression of the managed activity. Being governed does not relocate activity management to Region 5.
Management Monitoring	Observation and assessment of the current state of an activity for the purpose of determining progress, completion, fulfilment or impediments, so that the containing management or workflow activity can determine what should happen next.	May cause or inform progression, pause, retry, escalation, reassignment, response or completion. Observation alone is not Independent Assurance.
Independent Assurance	Evaluation of a governed subject against applicable obligations, constraints, controls, performance expectations, quality criteria or compliance checkpoints, using sufficient trustworthy evidence to establish an assurance conclusion.	Independently governs its evaluation and conclusion; does not progress or manage its subject.

Management Monitoring evaluates activity state in order to progress or manage the activity. Independent Assurance evaluates a governed subject against governing criteria in order to establish an assurance conclusion.

Evidence Distinctions

Evidence production does not itself perform assurance. Security, privacy, clinical, operational and information-management capabilities may produce trustworthy evidence used by Health Service Assurance without becoming assurance capabilities.

RAD source listing (presentation only; no inferred ancestry).

Provenance $\neq$ Assurance

Audit information / AuditEvent $\neq$ Assurance

Operational monitoring $\neq$ Assurance

Workflow progression $\neq$ Assurance

Business Outcome $\neq$ Assurance Outcome

Workflow completion $\neq$ Assurance

Incident $\neq$ Assurance Finding

Incident Response $\neq$ Assurance

Risk $\neq$ Assurance Finding

Risk ≠ Assurance Outcome

Subject-Specific Governance

Subject-specific governance remains with the capability responsible for that subject where governance is intrinsic to managing the subject itself.

Health Products & Clinical Technology (05), Standards, Semantics & Reference Governance (10), Security, Privacy & Digital Trust (12), and Workforce & Organisational Capability (15) retain their existing governance semantics and regional placement. Capability 12's compliance auditing and audit-record production do not, merely by that wording, establish independent assurance. Region 5 concerns governance and independent assurance as primary responsibilities.

Classification Reconciliation and Rationale

CT1	Previous → reconciled classification	Semantic rationale
05 Health Products & Clinical Technology	Reference → Harmonia-Relevant	Device integration provision/data integration and service-relevant device context materially affect information exchange and care enablement. Supply-chain and maintenance authority remain external.
07 Community Health & Wellbeing	Reference → Harmonia-Relevant	The established preventative programme/ cohort distinctions materially frame information, notification and care enablement. Wider public-health enterprise activity is not assigned to Harmonia.
08 Population Health & Health-System Planning	Adjacent → Harmonia-Relevant	Formal notification and governed population information materially support required health activity; planning remains externally governed and is not Core through context consumption.
13 Health Research & Innovation	Adjacent → Harmonia-Relevant	Governed provision of existing information for an authorised research purpose is material support, not passive context or ownership of trials/research. Innovation evaluation does not acquire a separate platform assignment.
14 Health Service Direction & Stewardship	Reference → Harmonia-Relevant	Service needs, viability, dependencies and alignment facts materially frame Harmonia's service information and derivation requirements. This does not assign executive/service-management authority or establish a future BEC.
15 Workforce & Organisational Capability	Harmonia-Relevant retained	Professional standing, privilege, coverage and organisational/service-offering facts materially constrain care enablement. Directory support does not make workforce/service governance Core.
16 Corporate Resources & Enterprise Services	Reference → Harmonia-Relevant	Required clinical activity/utilisation evidence is material financial/charging support. Corporate support context also matters, but contextual consumption alone does not classify every support CT3 Relevant or Core.

CT1	Previous → reconciled classification	Semantic rationale
17 Health Service Governance	Unresolved retained	The normative and compliance-definition responsibilities are established, but material platform participation/ enablement is not yet sufficiently assigned to distinguish Adjacent from Relevant/Core.
18 Health Service Assurance	Unresolved → Harmonia-Relevant	Already-approved independent governed assurance materially supports service confidence. Harmonia does not own macro-level clinical/service delivery assurance, so the whole BC is not classified Core.
01–04, 06, 09–12	Established assignments retained	Their material enabling/direct platform responsibilities remain established; organisational definitions do not turn clinical judgement, consent authority or enterprise decisions into software responsibilities. Core is bounded to assigned platform activity.

Unresolved Business Capability Matters

OI-BC-001 — Medication Compounding and Preparation

Unresolved. Whether compounding/specialised preparation warrants a distinct CT3 beneath Medication & Pharmacy Services is neither established nor rejected. Absence does not mean subsumption by Prescription Medication Dispensing, irrelevance, exclusion or representation by another Capability. Adjudication must preserve peer completeness and the R1/R2 materiality test.

OI-BC-002 — Health Service Operational Readiness Placement

The identified semantic concern asks: **Can this Health Service Offering actually be delivered from this operational context?** It establishes collective readiness from workforce, administration, spaces, equipment/devices, materials, utilities, ICT/communications and other dependencies without owning them. Ongoing site/service readiness and readiness for a session are presently exercises of the same concern; session-level exercise does not establish another Capability.

Its provisional name/identified identity and boundary are retained, but **CT2 parent, CT1 ancestry and canonical placement remain unresolved**. No complete CT1→CT2→CT3 path is asserted. Offering defined/temporally available $\neq$ collectively ready; individual resource ready $\neq$ service ready; care-place readiness $\neq$ service readiness; readiness oversight $\neq$ clinical execution or supporting-resource ownership. It SHALL NOT be placed beneath Health Service Alignment, Healthcare Service Offering Management, Care Scheduling & Capacity Management, Corporate Support Services, Health Product Management or another convenient parent without explicit adjudication.

Other Preserved Uncertainties and Canonical Conflicts

- Clinical Determination and Individual Care Planning have no established CT3 set in the supplied decisions. Clinical Intervention's three-child candidate remains provisional; DN-BC-010's taxonomy question is not closed by the queue.
- DN-BC-015 defines **Clinical Device Maintenance Management** as maintaining or restoring acceptable device condition through maintenance, testing, calibration, inspection, fault

remediation and repair, distinct from monitoring and overall lifecycle disposition. Its Working Decision and closing guardrail nevertheless specify seven CT3s omitting Maintenance. Its semantics are preserved, but its inclusion/tier/ancestry requires adjudication; no eighth child is manufactured.

- **Health Information Sharing Programme Management** is named by DN-BC-027's spelling correction, but that note supplies no precise tier/parent assertion. Its name is retained without constructing ancestry or an additional definition.
- **Health Information Exchange** is the approved BC CT2 name and an existing BEC name. Their identities/meanings remain distinct. This conflicts with CM-R05 and the BC/BEC naming guardrail; neither a BEC rename nor an identity merger is authorised here. The naming point remains stopped pending adjudication.
- Generic Continuous Improvement placement, external device/safety/quality monitoring providers, the possible Identity Assurance Representation BEC name and general UI/ presentation enablement remain unestablished/parked. No standards or other peer is added merely for symmetry. Other undecomposed enterprise scope remains explicitly unmodelled.

Semantic Relationships and Downstream Boundary

Business capabilities may provide evidence, context, identified events or established information to other business capabilities while retaining their own outcomes and source authority. Unless explicitly required otherwise, such provision is contextual rather than a universal dependency. **Information consumed $\neq$ ownership transferred; context supplied $\neq$ target outcome established.** These relationships are not decomposition, workflow order, runtime invocation, Service identity or application dependency. Any RADS relationship must preserve the exact meaning; where no governed type fits, preserve it as unresolved rather than substitute a stronger convenient type.

Established BC Evidence and Context Relationships

The following relationships preserve the adjudicated business meaning. They do not require every source instance to produce an input for every target instance, or make one capability perform another. A conditional relationship remains conditional. No named Business Service or machine-readable relationship type is allocated by this table.

Source	Recipient	Established meaning
Population Health Surveillance Management	Population Health Longitudinal Assessment; Population Health Event Identification	Supplies the population, concern and purpose of managed surveillance.
Population Health Longitudinal Assessment	Population Health Event Identification	Supplies trends/change evidence that may support recognition of a significant event.
Population Health Event Identification	Notifiable Health Event Management	Supplies an identified event for consideration under applicable notification obligations.
Population Health Longitudinal Assessment; Population Health Surveillance Management; Population Health Event Identification; Notifiable Health Event Management	Population Health Reporting	Supply their established assessments, surveillance state, events and notification information for reporting.
Population Health Longitudinal Assessment; Population Health Event Identification	Preventative Health Problem Identification	Supply population-health evidence/ identified concerns without determining the preventative response.

Source	Recipient	Established meaning
Population Health Longitudinal Assessment	Preventative Health Cohort Management	Supplies population characteristics relevant to cohort definition, stratification or review.
Population Health Event Identification	Preventative Health Cohort Management	May supply event context for identifying the population requiring preventative attention.
Individual Care Delivery	Population Health Surveillance	Individual information may contribute population evidence without becoming a population conclusion.
Population Health Surveillance	Individual Care Delivery	Population findings may inform individual care without establishing an Individual Clinical Determination.
Population Health Surveillance	Clinical Quality Management; Clinical Safety Management	May supply evidence relevant to their distinct assessments or risk identification; their information may also contribute population evidence.
Population Health Longitudinal Assessment; Population Health Event Identification	Health Service Demand Assessment	Supply population state/change or identified-event evidence relevant to expected service demand.
Health Service Demand Assessment	Health Service Capacity Planning	Supplies assessed demand evidence relevant to required capacity.
Health Service Capacity Planning	Health Service Distribution Planning	Supplies planned-capacity requirements relevant to the service footprint.
Operational Care Capacity Management	Health Service Demand Assessment; Health Service Capacity Planning	May supply operational pressure/ utilisation and capacity experience as planning evidence.
Health Service Distribution Planning	Healthcare Service Offering Management	May supply planned-footprint context for identifiable offerings.
Standards, Semantics & Reference Governance	Health Information Structure Management	Supplies governed semantic artefacts for reference/binding while retaining their authority.
Health Information Structure Management	Health Information Data Management	Supplies the structure, references, constraints and bindings through which content is meaningful.
Health Information Data Management	Health Information Lifecycle Management; Health Information Catalogue and Index Management; Health Information Compartmentalisation; Health Information Data Correlation	Supplies the managed information subject/content for these distinct responsibilities.
Health Information Catalogue and Index Management	Health Information Data Retrieval	Supplies existence, discovery and location context.
Health Information Compartmentalisation	Health Information Data Retrieval	Supplies applicable compartment scope/ context without making an access decision.
Health Information Data Correlation	Health Information Data Retrieval	May supply relationships relevant to identifying related requested information.
Health Information Structure Management; Health Information Lifecycle Management	Health Information Data Retrieval	Supply interpretation and applicable current/historical/lifecycle context.
Clinical Knowledge Context Identification	Clinical Knowledge Context Extraction	Supplies the identified source/context for extraction.
Clinical Knowledge Context Extraction	Clinical Knowledge Information Formulation	Supplies relevant extracted material for formulation.

Source	Recipient	Established meaning
Clinical Knowledge Information Formulation	Clinical Knowledge Concept Association; Clinical Knowledge Review & Approval Management	Supplies formulated knowledge for association and governed review.
Standards, Semantics & Reference Governance	Clinical Knowledge Concept Association	Supplies governed concepts/artefacts without transferring semantic authority.
Clinical Knowledge Concept Association	Clinical Knowledge Review & Approval Management	Supplies semantic-association context relevant to review.
Clinical Knowledge Review & Approval Management	Clinical Knowledge Publication & Sharing	Supplies knowledge whose applicable approval standing is established.
Clinical Knowledge Publication & Sharing	Individual Care Delivery	Supplies reusable knowledge; the individual-care capability establishes its clinical application/conclusion.
Health Information Management	Clinical Knowledge Context Identification	May supply source information containing potential reusable knowledge.

Material Traceability Requirements

Later derivation SHALL preserve established CT3 identity and the distinction between the Business purpose it supports and its owning BEC CT3 Feature frame. It SHALL NOT attach a Harmonia Feature directly to a BC CT3. Where the following contexts are material, traceability must preserve them without prescribing an Information Concept, representation, interface or implementation:

Concern	Context/evidence to preserve
Clinical risk	Evidence potentially indicating risk, the characterised risk, mitigation, active response and resulting state remain distinct.
Clinical quality	Underlying criterion authority; evidence source and purpose; criteria/evidence supporting an assessment; assessment authority distinct from its report and subsequent improvement.
Preventative health	Evidence supporting problem identification; target cohort; membership reasoning, inclusions/exclusions and applicable entry/exit or stratification. Programme type does not create another BC.
Population health	Population/cohort/geographical and temporal scope; evidence supporting assessment/event identification; notification obligation, recipient/authority, status and supporting event evidence.
Health-system planning	Demand basis, assumptions and scope; capacity requirements and distribution context; planned capacity distinguishable from operational evidence.
Health information	Applicable structures/reference targets/bindings and semantic authority; lifecycle state/history; catalogue versus content; compartment versus access decision; correlations and source provenance; retrieved information context.
Clinical knowledge	Source/extraction provenance and supporting formulation material; concept authority; review/approval state, conditions and history; publication preserving standing and limits; individual use distinct from general knowledge authority.

Later traceability must preserve source provenance, applicable context, temporal standing and these distinct business outcomes. Criteria do not replace governing requirements; reported representations do not originate the facts/conclusions they present. Programme-specific implementation does not establish another BC. Implementation reuse does not collapse BC peers or reverse-derive their meaning.

The separately approved assurance support derivation and B2.1 trace remain intact, with no extension to the newly reconciled CT3s. This reconciliation establishes no BEC ancestry, no

framing of the 137 Features, no new BC↔BEC mapping, no allocation of Canonical IDs/local codes and no Domain05 B2.2 work. The 69 unresolved enabling descriptions remain unresolved. Those are subsequent phases.

- Business Enabling Capabilities
- Capability decomposition and derivation model
- Historical five-region reconciliation
- Business Architecture reference candidate

38. Canonical Business Enabling Capability Model & Features

RAD source: `docs/markdown/02-strategy/capabilities/business-enabling-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Business Enabling Capability Model & Features

Overview & Architecture

The Business Enabling Capability model defines **what systems and information infrastructure must enable or provide in support of the healthcare enterprise**.

Where Business Capabilities define the high-level healthcare landscape (what the enterprise does), Business Enabling Capabilities specify the functional abilities systems must deliver to make healthcare delivery, administration, and operations possible.

Architectural Principles Governing this Model

1. **Authentic Healthcare Operating Contexts**: Capabilities are grouped into five authentic healthcare contextual views. They reflect genuine healthcare operating environments (e.g., Ward Operations, Order Administration, Patient Clinical Record), **not** low-level integration engine plumbing or middleware mechanics.
2. **Independence from Commercial Product Boundaries**: Capabilities are defined independently of commercial software product boundaries (such as PAS, EMR, LIS, RIS, HRMS, or FMIS). A commercial product may implement multiple capabilities, drawing "dotted lines" around parts of the model; the capability model remains orthogonal to vendor packaging.
3. **Atomic Feature Decomposition**: Within established Harmonia-Relevant and Harmonia-Core Capability contexts, system behaviour is decomposed into atomic **Features**—the smallest useful, testable, technology-neutral statements of system-enabled behaviour. Reference and Adjacent capabilities are intentionally not decomposed into features.
4. **Technology Neutrality**: No capability or feature is defined in terms of specific software products (HAPI FHIR, Infinispan, PostgreSQL, ActiveMQ Artemis, Camel, Netty, Vue, Spring) or runtime plumbing (threads, queues, ports, DDL).
5. **Architectural Scope Sufficiency**: The Business Enabling Feature catalogue provides the established R1.x/R2.x Feature baseline for the capabilities and contextual views presently decomposed. Completeness SHALL NOT be interpreted as requiring artificial Feature decomposition of capabilities for which no Feature-level decomposition has been architecturally established. Roadmap items (such as authoritative master-patient EMPI reconciliation) are explicitly demarcated as uncommitted Harmonia 3.x candidates.

Under approved G1 K9, the five views, catalogue numbering, indentation and decomposition presentation do not establish Architectural Element identity, CT1 / CT2 / CT3 assignment, ancestry or root status. The 137 established Features retain their FT type, names and identifiers. Affected complete Capability ancestry and structural Canonical IDs remain unresolved. Derivation may proceed from established ownership and responsibility without inventing intermediate elements.

The fixed Capability decomposition requires **CT1 → CT2 → CT3 → FT** for every fully reconciled Feature. Every Feature SHALL be framed by CT3 with CT2 and CT1 parents; tiers SHALL NOT

be skipped. A Feature cannot compensate for incomplete upstream Capability modelling. Existing Features without established CT3 frames remain valid evidence with Capability ancestry/decomposition requiring separate reconciliation; their current placement does not establish direct CT1/CT2 → FT permission. Neither the 69 unresolved enabling catalogue descriptions nor any other presentation entry becomes a CT3 Capability through this obligation. No Capability, tier assignment, Feature, local ID or complete Canonical ID is created or changed here.

The approved Health Service Assurance derivation additionally establishes **Assurance Design**, **Assurance Criteria Management** and **Governed Assurance** as Business Enabling Capabilities supporting BC-18. Their definitions and assurance principles are recorded below. Their contextual-view placement, relevance classification, Capability Tier, complete ancestry and structural Canonical IDs remain unresolved; they are not assigned through document position or lexical similarity. No additional Feature catalogue is derived.

The Five Contextual Views

The Business Enabling capabilities are structured across five contextual views:

RAD source listing (presentation only; no inferred ancestry).

1. ENTITY MANAGEMENT

   Governance and administration of healthcare entities throughout their lifecycle: Clients, Providers, Organisations, Locations, Services, Products, and Devices.

2. SERVICE ADMINISTRATION

   Administrative, logistical, and financial progression of care: Client Intake, Referrals, Encounters, Orders, Diagnostics, Care Plans, Clinical Messaging, and Billing Extraction.

3. SERVICE DELIVERY

   Systems enablement across clinical delivery settings: Primary Care, Acute Inpatient, Emergency, Surgical, Diagnostic, Medication, Allied Health, Community, and Remote/Virtual Care.

4. HEALTH SERVICE OPERATIONS

   Facility logistics, bed and care-place management, capacity, clinical resource coordination, work allocation and dispatch, patient transport, and discharge progression.

5. INTRINSIC / SHARED ENABLEMENT

   Reusable system capabilities: Longitudinal Clinical Record, Health Information Exchange, Information Access, Policy & Security Control, Collaboration, Workflow Coordination, and Presentation.

Architectural Scope Statement (Harmonia 1.x/2.x vs. 3.x Roadmap)

> The Business Enabling Feature catalogue provides the established R1.x/R2.x Feature baseline for the capabilities and contextual views presently decomposed. Completeness SHALL NOT be interpreted as requiring artificial Feature decomposition of capabilities for which no Feature-level decomposition has been architecturally established.

- **Established Coverage**: The model records established capability responsibilities and the 137-Feature baseline. BC-17 enablement and the assurance capabilities' unestablished contextual-view, tier, ancestry and Feature relationships remain explicit gaps; structural symmetry is not a completion requirement.

- **Sufficient Does Not Mean Frozen Forever**: Additional capabilities and features may be introduced where future Harmonia scope requires them.
- **Roadmap Demarcation — Patient Identity**:
  - **Harmonia 1.x/2.x Scope**: Includes cross-system identifier resolution, identifier correlation, identity alias association, identity provenance, identity federation across regional nodes, and governed administrative correction.
  - **Harmonia 3.x Roadmap Candidate**: Authoritative master-patient identity reconciliation, probabilistic matching algorithms, and Enterprise Master Person Index (EMPI) merge/unmerge engines remain an uncommitted Harmonia 3.x candidate. Harmonia 1.x/2.x relies on authoritative upstream PAS/MPI feeds rather than acting as the master clinical EMPI authority.

View 1: Entity Management

Entity Management encompasses systems enablement for identifying, governing, and managing core healthcare entities throughout their operational lifecycles.

RAD source listing (presentation only; no inferred ancestry).

1. Client Administration

1.1 Person Identity [Harmonia-Core]

1.2 Healthcare Subject [Harmonia-Core]

1.3 Client Relationship [Harmonia-Relevant]

1.4 Client Privacy [Harmonia-Core]

2. Provider Administration

2.1 Practitioner [Harmonia-Core]

2.2 Practitioner Role [Harmonia-Core]

2.3 Provider Relationship [Harmonia-Relevant]

2.4 Provider Engagement [Harmonia-Relevant]

3. Organisation Administration

3.1 Healthcare Organisation [Harmonia-Core]

3.2 Organisational Structure [Harmonia-Core]

3.3 Organisation Contact [Harmonia-Relevant]

4. Location Administration

4.1 Healthcare Location [Harmonia-Core]

4.2 Location Structure [Harmonia-Core]

4.3 Care-Place Definition [Harmonia-Core]

5. Health Service Administration

5.1 Healthcare Service Definition [Harmonia-Core]

5.2 Service Provision Relationship [Harmonia-Core]

5.3 Service Availability Definition [Harmonia-Core]

5.4 Service Eligibility Definition [Harmonia-Relevant]

6. Health Product Administration

6.1 Health Product Definition [Adjacent]

6.2 Product Formulary & Catalogue [Adjacent]

6.3 Product Batch & Serial Tracking [Reference]

7. Clinical Device Administration

7.1 Device Definition [Harmonia-Relevant]

7.2 Device Instance [Harmonia-Relevant]

7.3 Device Association [Harmonia-Relevant]

7.4 Device Connectivity [Harmonia-Relevant]

Detailed Capability & Feature Catalogue: Entity Management

1. Client Administration

Governs the representation, identification, and demographic context of individuals receiving care.

- **1.1 Person Identity** (`Harmonia-Core`)
  - *Definition*: Resolving, maintaining, and correlating identities and identifiers assigned to an individual across different authorities.
  - `FEAT-EM-01`: **Identifier Resolution**: Ingest and resolve external identifiers against registered issuing authorities and namespace systems.
  - `FEAT-EM-02`: **Cross-Authority Identifier Correlation**: Correlate disparate local identifiers (e.g., hospital MRN, clinic client ID, national IHI) belonging to the same individual.
  - `FEAT-EM-03`: **Identity Alias Association**: Maintain explicit associations between active identifiers and historical or demographic aliases without destructive overwriting.
  - `FEAT-EM-04`: **Governed Identity Correction**: Apply administrative identity updates, linked-record corrections, and split/merge notifications propagated from authoritative sources.
- **1.2 Healthcare Subject** (`Harmonia-Core`)
  - *Definition*: Managing the core healthcare subject context bound to clinical records and operational activities.
  - `FEAT-EM-05`: **Subject Context Binding**: Bind authenticated person identity to clinical encounters, orders, and observations as the enduring subject of care with verifiable association.
  - `FEAT-EM-06`: **Demographic Context Governance**: Manage essential demographic attributes (date of birth, sex, vital status) with full provenance and timestamping.
- **1.3 Client Relationship** (`Harmonia-Relevant`)
  - *Definition*: Representing relationships between the client and carers, guardians, or contacts.
  - `FEAT-EM-07`: **Next-of-Kin & Guardian Association**: Associate nominated emergency contacts, legal guardians, and care agents with the client record.
  - `FEAT-EM-08`: **Relationship Validity Tracking**: Maintain effective date intervals and authority scopes for nominated client relationships.
- **1.4 Client Privacy** (`Harmonia-Core`)
  - *Definition*: Enforcing client-level privacy preferences, consent directives, and confidentiality markers.
  - `FEAT-EM-09`: **Consent Directive Evaluation**: Evaluate client consent directives against requested information disclosure and exchange actions.
  - `FEAT-EM-10`: **Confidentiality Flag Enforcement**: Enforce restricted access flags (e.g., VIP, sensitive diagnosis, restricted record) across query and presentation interfaces.

2. Provider Administration

Governs healthcare practitioners, their professional qualifications, operational roles, and organisational engagements.

- **2.1 Practitioner** (`Harmonia-Core`)
  - *Definition*: Identifying and verifying individual healthcare professionals.

  - `FEAT-EM-11`: **National Practitioner Verification**: Ingest, validate, and verify national practitioner identifiers (e.g., HPI-I) and professional registration status.
  - `FEAT-EM-12`: **Practitioner Profile Governance**: Maintain core professional demographic details, qualifications, and active practice status.
- **2.2 Practitioner Role** (`Harmonia-Core`)
  - *Definition*: Modeling the formal role, specialty, and operational capacity of a practitioner at a given organisation or location.
  - `FEAT-EM-13`: **Role & Specialty Modeling**: Define and associate clinical specialties, practice roles, and service capacities for a practitioner.
  - `FEAT-EM-14`: **Electronic Endpoint Binding**: Associate validated communication endpoints (secure messaging addresses, direct routing queues) with specific practitioner roles.
- **2.3 Provider Relationship** (`Harmonia-Relevant`)
  - *Definition*: Managing organizational affiliations and multidisciplinary team relationships.
  - `FEAT-EM-15`: **Practitioner-Organisation Affiliation**: Maintain temporal affiliations between practitioners and healthcare organizations.
- **2.4 Provider Engagement** (`Harmonia-Relevant`)
  - *Definition*: Capturing terms of service and clinical privilege agreements.
  - `FEAT-EM-16`: **Clinical Privileging Record**: Track active clinical admitting and practice privileges within healthcare facilities.

3. Organisation Administration

Governs healthcare delivery organizations, facilities, and network structures.

- **3.1 Healthcare Organisation** (`Harmonia-Core`)
  - *Definition*: Authoritative identification and profiling of legal and operational healthcare entities.
  - `FEAT-EM-17`: **National Organisation Verification**: Validate national organization identifiers (e.g., HPI-O) and legal entity classifications.
  - `FEAT-EM-18`: **Organisation Profile Management**: Govern directory profiles, operational status, and published service descriptions.
- **3.2 Organisational Structure** (`Harmonia-Core`)
  - *Definition*: Managing parent-child hierarchies, departments, and corporate relationships.
  - `FEAT-EM-19`: **Hierarchical Structure Navigation**: Model, traverse, and resolve nested departmental, divisional, and regional healthcare networks.
- **3.3 Organisation Contact** (`Harmonia-Relevant`)
  - *Definition*: Maintaining verified administrative and clinical communication channels.
  - `FEAT-EM-20`: **Directory Contact Management**: Manage verified phone, email, and administrative contact channels for healthcare facilities.

4. Location Administration

Governs the physical spaces, facilities, and care environments where healthcare is delivered.

- **4.1 Healthcare Location** (`Harmonia-Core`)
  - *Definition*: Identifying and profiling geographic locations, physical campuses, and health facilities.
  - `FEAT-EM-21`: **Location Geospatial & Address Resolution**: Validate physical addresses, geospatial coordinates, and location operational status.
- **4.2 Location Structure** (`Harmonia-Core`)
  - *Definition*: Modeling the physical architecture of healthcare campuses.
  - `FEAT-EM-22`: **Physical Hierarchy Modeling**: Represent campus, building, wing, floor, and ward architectural relationships.

- **4.3 Care-Place Definition** (`Harmonia-Core`)
  - *Definition*: Modeling the atomic care delivery places (beds, bays, treatment chairs, surgical tables).
  - `FEAT-EM-23`: **Atomic Care-Place Modeling**: Define specific beds, care-bays, and examination rooms with their physical capabilities (e.g., telemetry-capable, negative-pressure).

5. Health Service Administration

Governs the healthcare services offered by providers and organizations.

- **5.1 Healthcare Service Definition** (`Harmonia-Core`)
  - *Definition*: Specifying clinical and administrative health services offered to the public or clinical referrers.
  - `FEAT-EM-24`: **Service Catalogue Publishing**: Publish structured healthcare service definitions, clinical specialties, and intake criteria to regional directories.
- **5.2 Service Provision Relationship** (`Harmonia-Core`)
  - *Definition*: Binding health services to specific organizations, locations, and practitioner roles.
  - `FEAT-EM-25`: **Service-Location-Provider Binding**: Correlate which organization provides a named health service at which physical location with which practitioner roles.
- **5.3 Service Availability Definition** (`Harmonia-Core`)
  - *Definition*: Modeling regular operating hours, exceptional closures, and emergency availability.
  - `FEAT-EM-26`: **Operating Schedule Specification**: Define temporal availability, clinic operating hours, and on-call service windows.
- **5.4 Service Eligibility Definition** (`Harmonia-Relevant`)
  - *Definition*: Capturing referral criteria, catchment boundaries, and clinical eligibility.
  - `FEAT-EM-27`: **Eligibility & Catchment Specification**: Define clinical referral thresholds and geographic catchment boundaries.

6. Health Product Administration

Governs medications, blood products, implants, and consumable health goods.

- **6.1 Health Product Definition** (`Adjacent`): Classification of pharmaceutical and consumable products.
- **6.2 Product Formulary & Catalogue** (`Adjacent`): Management of approved facility medication formularies.
- **6.3 Product Batch & Serial Tracking** (`Reference`): Supply chain batch and lot management. *(No features decomposed for Reference/Adjacent capabilities).*

7. Clinical Device Administration

Governs medical equipment, patient monitors, diagnostic instruments, and telehealth devices.

- **7.1 Device Definition** (`Harmonia-Relevant`)
  - *Definition*: Specifying medical equipment types, models, and regulatory classifications.
  - `FEAT-EM-28`: **Device Type Specification**: Catalog medical device models, regulatory approvals, and capabilities.
- **7.2 Device Instance** (`Harmonia-Relevant`)
  - *Definition*: Tracking physical device hardware instances and serial numbers.
  - `FEAT-EM-29`: **Device Serial Tracking**: Ingest and maintain unique device identifier (UDI) records and serial numbers.
- **7.3 Device Association** (`Harmonia-Relevant`)

  - *Definition*: Associating clinical devices with patients, beds, or rooms.
  - `FEAT-EM-30`: **Device-Patient/Location Assignment**: Track dynamic association between a telemetry monitor, patient, and physical bed.
- **7.4 Device Connectivity** (`Harmonia-Relevant`)
  - *Definition*: Registering communication protocols and network endpoints for medical devices.
  - `FEAT-EM-31`: **Device Endpoint Registration**: Register communication interfaces and security credentials for diagnostic telemetry feeds.

View 2: Service Administration

Service Administration encompasses systems enablement for the operational, administrative, logistical, and financial progression of care.

RAD source listing (presentation only; no inferred ancestry).

1. Client-Centred Administration [Harmonia-Relevant]
2. Referral Administration [Harmonia-Relevant]
3. Scheduling Administration [Harmonia-Relevant]
4. Episode & Encounter Administration [Harmonia-Core]
5. Order Administration [Harmonia-Core]
6. Diagnostic Administration [Harmonia-Core]
7. Medication Administration [Harmonia-Relevant]
8. Procedure Administration [Harmonia-Relevant]
9. Clinical Record Administration [Harmonia-Core]
10. Care Plan Administration [Harmonia-Relevant]
11. Clinical Communication Administration [Harmonia-Core]
12. Funding & Coverage Administration [Adjacent]
13. Clinical Charging & Claims Administration [Adjacent]

Critical Domain Distinctions: Service Administration

Distinction: Referral vs. Order

- **Referral Administration**: A **Referral** formally requests another care participant or service provider to assess, manage or assume some degree of clinical care responsibility. Referral intent, disposition, responsibility actually assumed and Transfer of Care are distinct. Referral may lead to shared or assumed responsibility or transfer of ongoing care, but does not itself establish acceptance, delivery, assumption or Transfer of Care, or universally require an Appointment, Encounter or Order.
- **Order Administration**: An **Order** requests a defined, bounded **fulfilment activity** (e.g., execute a diagnostic test, dispense a medication, perform a procedure) and strictly supports a **closed-loop progression** returning an outcome to the requester.

Distinction: Technical ACK vs. Business ACK

- **Technical Delivery Acknowledgement**: A technical acknowledgement establishes only the acceptance state defined by the applicable interaction contract. It SHALL NOT be interpreted as durable acceptance, committed business state, successful processing or successful business disposition unless that meaning is explicitly established by the governing

contract. HTTP 200 does not universally establish successful business disposition, and HTTP 202 does not universally establish durable queue acceptance; either may have stronger meaning under an explicit contract.
- **Business Acknowledgement**: A clinical/operational message generated by the receiving application or clinical staff confirming that the order or referral has been reviewed, accepted for execution, scheduled, or rejected. Technical Delivery Acknowledgement $\neq$ Business Acknowledgement

Acknowledgement of a handoff is not, by itself, evidence of eventual downstream business outcome. REQ-FND-001 continues to require the applicable durable acceptance boundary before Harmonia positively acknowledges ingress acceptance.

Business acknowledgement is contextual and may communicate acceptance, rejection or another explicit disposition; it does not establish a universal reviewed → accepted → scheduled progression or transfer originating authority.

Order Closed-Loop Progression Semantics

All order-related capabilities must support the end-to-end closed loop: Destination → Route → Deliver → Acknowledge → Progress → Complete → Outcome

Detailed Capability & Feature Catalogue: Service Administration

1. Client-Centred Administration (`Harmonia-Relevant`)

- **1.1 Client Intake & Demographic Update**
  - `FEAT-SA-01`: **Demographic Update Ingestion**: Ingest and validate patient admission and registration updates from external PAS systems.
  - `FEAT-SA-02`: **Demographic Change Distribution**: Fan out verified client demographic updates to downstream clinical subsystems.

2. Referral Administration (`Harmonia-Relevant`)

- **2.1 Referral Intake & Progression**
  - `FEAT-SA-03`: **Referral Document Ingestion**: Ingest structured clinical referral messages across integration boundaries.
  - `FEAT-SA-04`: **Referral Supporting Information**: Collate and associate supporting clinical documentation, diagnostic history, and patient context with referral requests.
  - `FEAT-SA-05`: **Referral Status & Outcome Tracking**: Track referral operational progression and communicate referral outcomes (acceptance, decline, or redirection) to referring providers.

3. Scheduling Administration (`Harmonia-Relevant`)

- **3.1 Appointment & Booking Administration**
  - `FEAT-SA-06`: **Appointment Notification Ingestion**: Ingest scheduled appointment events from departmental scheduling systems.
  - `FEAT-SA-07`: **Appointment Status Synchronization**: Propagate booking cancellations, reschedules, and attendances across care coordination systems.

4. Episode & Encounter Administration (`Harmonia-Core`)

- **4.1 Encounter Lifecycle Management**

- FEAT-SA-08: **Encounter Context Creation**: Create and maintain authoritative encounter tracking contexts for hospital admissions, outpatient visits, and community contacts.
- FEAT-SA-09: **Encounter State Progression**: Track encounter status transitions (PLANNED → ARRIVED → IN_PROGRESS → ON_LEAVE → DISCHARGED).
- FEAT-SA-10: **Encounter Bed/Location Association**: Dynamically maintain the association between an encounter, care team, and physical bed or ward location.

5. Order Administration (Harmonia-Core)

- **5.1 Closed-Loop Order Progression**
  - FEAT-SA-11: **Order Request Ingestion**: Ingest clinical diagnostic, medication, and procedure order requests with validated ordering clinician context.
  - FEAT-SA-12: **Order Destination Resolution & Routing**: Resolve fulfillment endpoints and reliably route orders to performing service systems.
  - FEAT-SA-13: **Order Closed-Loop Progression Tracking**: Track order progression across destination determination, routing, delivery, receipt acknowledgement (distinguishing technical delivery acknowledgement from business acknowledgement), execution progress, completion, and outcome association.
  - FEAT-SA-14: **Order Cancellation & Modification Coordination**: Coordinate order cancellation and amendment requests, ensuring synchronization with performing systems.

6. Diagnostic Administration (Harmonia-Core)

- **6.1 Diagnostic Workflow Management**
  - FEAT-SA-15: **Diagnostic Request Correlation**: Correlate diagnostic test orders with collected clinical specimens and patient encounters.
  - FEAT-SA-16: **Diagnostic Report Ingestion & Binding**: Ingest finalized laboratory and radiology reports, verifying content integrity and binding them to the parent order.
  - FEAT-SA-17: **Diagnostic Report Distribution**: Distribute finalized laboratory, pathology, and imaging reports to ordering clinicians and designated care teams.

This Feature retains its finalised-report responsibility. Broader Domain03 Diagnostic Administration behaviour may also handle preliminary, corrected and addendum/correction information without redefining this narrower Feature.

7. Medication Administration (Harmonia-Relevant)

- **7.1 Medication Order & Dispense Management**
  - FEAT-SA-18: **Medication Order Ingestion**: Ingest inpatient and discharge prescription orders.
  - FEAT-SA-19: **Dispense Event Tracking**: Ingest medication dispense verification records from pharmacy management systems.
  - FEAT-SA-20: **Medication Administration Record (MAR) Ingestion**: Ingest point-of-care medication administration events.

8. Procedure Administration (Harmonia-Relevant)

- **8.1 Procedure & Surgical Administration**
  - FEAT-SA-21: **Procedure Booking Ingestion**: Ingest scheduled surgical case bookings and procedural requests.
  - FEAT-SA-22: **Procedural Documentation Ingestion**: Ingest operative reports, anesthesia records, and post-procedure summaries.

9. Clinical Record Administration (`Harmonia-Core`)

- **9.1 Clinical Documentation Lifecycle**
  - `FEAT-SA-23`: **Clinical Document Ingestion**: Ingest structured and unstructured clinical documents (discharge summaries, consult notes, letters).
  - `FEAT-SA-24`: **Document Versioning & Supersession**: Manage document classification, author attribution, amendment attribution, versioning, and supersession or withdrawal without destructive data loss.
  - `FEAT-SA-25`: **Document Metadata Indexing**: Index clinical document metadata (author, specialty, encounter, date, confidentiality) for rapid discovery.

10. Care Plan Administration (`Harmonia-Relevant`)

- **10.1 Care Plan Coordination**
  - `FEAT-SA-26`: **Care Plan Ingestion & Distribution**: Ingest multidisciplinary care plans and syndicate updates to authorized care team members.

11. Clinical Communication Administration (`Harmonia-Core`)

- **11.1 Secure Clinical Communication**
  - `FEAT-SA-27`: **Clinical Communication Distribution**: Distribute secure clinical communications between verified healthcare providers and designated care team endpoints.
  - `FEAT-SA-28`: **Delivery Acknowledgement Tracking**: Correlate recipient delivery and read receipts with outbound clinical communications, distinguishing transport delivery from recipient acknowledgement. Receipt/read evidence does not by itself establish business acceptance, clinical review, incorporation, approval, finality or originating authority.
  - `FEAT-SA-29`: **Communication Audit Logging**: Maintain an enduring, non-PHI, tamper-evident log of all clinical communication transactions.

12. Funding & Coverage Administration (`Adjacent`)

- Ingestion of patient insurance coverage and eligibility flags. *(No features decomposed)*.

13. Clinical Charging & Claims Administration (`Adjacent`)

- Export of clinical activity summaries and charging records to billing systems. *(No features decomposed)*.

View 3: Service Delivery

Service Delivery encompasses systems enablement across genuine healthcare delivery settings and clinical care delivery contexts.

These established Business Enabling contextual entries are not the CT2 decomposition of 01 Individual Care Delivery. The Business Capability's six CT2 abilities remain independent of these contexts under B2-R05 and B2-R06.

RAD source listing (presentation only; no inferred ancestry).

1. Clinical Assessment [Adjacent]
2. Primary Care [Harmonia-Relevant]
3. Acute Care [Harmonia-Relevant]

4. Emergency Care [Harmonia-Relevant]
5. Inpatient Care [Harmonia-Relevant]
6. Ambulatory Care [Adjacent]
7. Surgical & Procedural Care [Adjacent]
8. Diagnostic Services [Harmonia-Relevant]
9. Therapeutic Services [Adjacent]
10. Medication Therapy [Harmonia-Relevant]
11. Allied Health [Adjacent]
12. Preventive Care [Harmonia-Relevant]
13. Rehabilitation [Adjacent]
14. Community Care [Harmonia-Relevant]
15. Outreach Care [Harmonia-Relevant]
16. Remote & Virtual Care [Harmonia-Relevant]

Architectural Principles: Service Delivery

1. **Heterogeneous Axes Preserved**: The 16 Service Delivery capabilities represent heterogeneous clinical settings (acute, emergency, primary), modalities (virtual, outreach), and specialties (diagnostic, therapeutic). They are deliberately preserved as authentic healthcare contexts rather than normalized into artificial abstractions.
2. **Zero Harmonia-Core in Service Delivery**: No Service Delivery capability is classified as `Harmonia-Core`. Harmonia provides integration, transport, state management, and notification; **Harmonia does not practice medicine or deliver clinical therapy**.
3. **Canonical Enabling Pattern for Relevant Features**: Provide Context → Receive Clinical State/Outcome → Correlate → Preserve Provenance → Make Available → Enable Next Activity

Detailed Capability & Feature Catalogue: Service Delivery

2. Primary Care (`Harmonia-Relevant`)

- **2.1 Primary Care Coordination**
  - `FEAT-SD-01`: **General Practice Shared Record Ingestion**: Ingest shared health summaries, encounter notes, and immunization records from primary care practice management systems.
  - `FEAT-SD-02`: **Primary Care Notification Dispatch**: Dispatch emergency department arrival, acute admission, and discharge notifications to the patient's registered general practitioner.

3. Acute Care (`Harmonia-Relevant`)

- **3.1 Acute Care Monitoring & Event Coordination**
  - `FEAT-SD-03`: **Acute Clinical State Event Capture**: Ingest clinical deterioration notifications and acute clinical status updates.
  - `FEAT-SD-04`: **Acute Care Clinical Context Provision**: Provide consolidated longitudinal history and active medication lists to acute care clinicians at point of triage.

4. Emergency Care (`Harmonia-Relevant`)

- **4.1 Emergency Care Progression**
  - `FEAT-SD-05`: **Emergency Department Arrival & Triage Ingestion**: Ingest emergency presentation, triage category, and chief complaint records.
  - `FEAT-SD-06`: **Emergency Care Cross-Facility Correlation**: Correlate incoming emergency presentations with recent regional admissions, prior discharge summaries, and known allergy profiles.

5. Inpatient Care (`Harmonia-Relevant`)

- **5.1 Multidisciplinary Inpatient Care Coordination**
  - `FEAT-SD-07`: **Inpatient Rounding Context Provision**: Aggregate diagnostic results, current medication administration records, and nursing notes for multidisciplinary inpatient rounds.
  - `FEAT-SD-08`: **Inpatient Clinical Progression Tracking**: Capture inpatient clinical milestone completions (e.g., medical stability achieved, allied health clearance).

8. Diagnostic Services (`Harmonia-Relevant`)

- **8.1 Diagnostic Service Integration**
  - `FEAT-SD-09`: **Laboratory & Imaging Result Distribution**: Reliably distribute structured diagnostic reports (pathology, radiology) to ordering clinicians and designated secondary recipients.
  - `FEAT-SD-10`: **Diagnostic History Consolidation**: Consolidate historical diagnostic trends (e.g., cumulative laboratory results) across multiple regional providers into an integrated view.

10. Medication Therapy (`Harmonia-Relevant`)

- **10.1 Medication Therapy Provision and Monitoring**
  - `FEAT-SD-11`: **Medication History Reconciliation Ingestion**: Ingest reconciled medication histories compiled during clinical transitions of care.
  - `FEAT-SD-12`: **Medication Administration Tracking**: Distribute medication therapy orders, dispensing updates, and administration records across clinical care boundaries.

12. Preventive Care (`Harmonia-Relevant`)

- **12.1 Screening and Immunisation Enablement**
  - `FEAT-SD-13`: **Immunisation Event Submission**: Ingest point-of-care immunisation administration records and syndicate to national immunization registers.
  - `FEAT-SD-14`: **Screening Recall Notification Distribution**: Distribute clinical screening reminders and recall notices to designated healthcare providers.

14. Community Care (`Harmonia-Relevant`)

- **14.1 Community Care Coordination**
  - `FEAT-SD-15`: **Community Encounter Ingestion**: Ingest home visit notes, community nursing observations, and social support assessments.
  - `FEAT-SD-16`: **Community Care Plan Synchronization**: Synchronize community care plan updates between hospital outreach teams and community health organizations.

15. Outreach Care (`Harmonia-Relevant`)

- **15.1 Outreach Care Continuity**
  - `FEAT-SD-17`: **Outreach Visit Context Provision**: Package and deliver portable clinical summaries and offline-capable care summaries for remote outreach clinics.
  - `FEAT-SD-18`: **Outreach Encounter Outcome Ingestion**: Ingest and reconcile clinical documentation gathered during rural and remote outreach visits.

16. Remote & Virtual Care (`Harmonia-Relevant`)

- **16.1 Remote Patient Monitoring**
  - `FEAT-SD-19`: **Remote Telemetry Ingestion**: Ingest home patient monitoring data, biometric telemetry, and patient-reported outcome measures (PROMs).
  - `FEAT-SD-20`: **Virtual Care Session Context Binding**: Associate synchronous virtual/telehealth consultations with the longitudinal health record and active care episode.

(Capabilities 1 Clinical Assessment, 6 Ambulatory Care, 7 Surgical & Procedural Care, 9 Therapeutic Services, 11 Allied Health, and 13 Rehabilitation remain Adjacent or Reference and are not decomposed into atomic features).

View 4: Health Service Operations

Health Service Operations encompasses systems enablement for the operational management, logistics, capacity, and resource coordination of healthcare facilities and services.

RAD source listing (presentation only; no inferred ancestry).

1. Clinic & Practice Operations [Harmonia-Relevant]
2. Ward Operations [Harmonia-Relevant]
3. Theatre Operations [Harmonia-Relevant]
4. Emergency Department Operations [Harmonia-Relevant]
5. Outpatient Operations [Harmonia-Relevant]
6. Bed & Care-Place Management [Harmonia-Core]
7. Clinical Resource Management [Harmonia-Relevant]
8. Service Capacity Management [Harmonia-Relevant]
9. Clinical Workforce Management [Adjacent]
10. Clinical Rostering [Adjacent]
11. Mobile Staff Management [Harmonia-Relevant]
12. On-Call Management [Harmonia-Relevant]
13. Work Allocation & Dispatch [Harmonia-Core]
14. Clinical Training & Education [Reference]
15. Clinical Competency Management [Reference]
16. Clinical Qualification Management [Harmonia-Relevant]
17. Clinical Supervision [Reference]
18. Patient Transport [Harmonia-Core]
19. Clinical Logistics Coordination [Harmonia-Relevant]
20. Discharge Management [Harmonia-Core]

Architectural Principles: Health Service Operations

1. **Facility Logistics, Not IT Machinery**: Health Service Operations reflects real-world health facility logistics and coordination (wards, beds, staff allocation, patient transfer); it must **never** be redefined or reinterpreted as protocol mediation, message queuing, deduplication, or integration software machinery.
2. **Healthcare Service Context Rule**:

   > **Where operational activity occurs in the context of a Healthcare Service, that service context must be explicitly associated with the activity and state rather than inferred solely from practitioner, organisation, or location.**

1. **General Operational Progression Pattern**: Event → Entity Context → Required Activity → Assignment → Dispatch → ACK → Progress → Outcome → Entity State Change → Next Activity

Detailed Capability & Feature Catalogue: Health Service Operations

1. Clinic & Practice Operations (`Harmonia-Relevant`)

- **1.1 Session & Queue Coordination**
  - `FEAT-HSO-01`: **Clinic Session Tracking**: Ingest and track clinic operating session start, delay, and completion states.
  - `FEAT-HSO-02`: **Operational Queue Progression**: Track client queue progression through reception, waiting area, consultation, and billing.

2. Ward Operations (`Harmonia-Relevant`)

- **2.1 Ward Logistics Coordination**
  - `FEAT-HSO-03`: **Ward Patient Flow Coordination**: Coordinate inpatient ward occupancy, care-place allocation, and patient bed status across care facilities.
  - `FEAT-HSO-04`: **Clinical Handover Context Assembly**: Collate current ward patient lists, active medical concerns, and pending tasks for nursing shift handovers.

3. Theatre Operations (`Harmonia-Relevant`)

- **3.1 Perioperative Logistics**
  - `FEAT-HSO-05`: **Operating Theatre Case Progression**: Track surgical case state transitions (`CALLED` → `IN_THEATRE` → `ANAESTHETISED` → `INCISION` → `CLOSED` → `RECOVERY`).
  - `FEAT-HSO-06`: **Perioperative Resource Notification**: Dispatch alerts to portering, sterilization, and recovery teams based on surgical case progression milestones.

4. Emergency Department Operations (`Harmonia-Relevant`)

- **4.1 ED Operational Coordination**
  - `FEAT-HSO-07`: **ED Bed & Bay Tracking**: Track occupancy and status of emergency cubicles, resuscitation bays, and short-stay beds.
  - `FEAT-HSO-08`: **ED Length of Stay Tracking**: Monitor operational wait times and length-of-stay milestones, generating operational escalation notifications.

5. Outpatient Operations (`Harmonia-Relevant`)

- **5.1 Outpatient Flow Enablement**
  - `FEAT-HSO-09`: **Outpatient Arrival Registration**: Ingest self-service check-in and clinic arrival events, notifying treating clinicians.
  - `FEAT-HSO-10`: **Post-Clinic Order Coordination**: Capture follow-up booking and diagnostic order requirements generated at clinic completion.

6. Bed & Care-Place Management (`Harmonia-Core`)

- **6.1 Bed State & Turnover Management**
  - `FEAT-HSO-11`: **Bed Availability Tracking**: Maintain operational status of all registered beds and care-places (`VACANT`, `OCCUPIED`, `BLOCKED`, `RESERVED`, `DIRTY`, `CLEANING`, `CLEAN`).
  - `FEAT-HSO-12`: **Bed State Progression**: Progress bed states in response to clinical and environmental events (e.g., patient discharge automatically transitions bed to `DIRTY`).
  - `FEAT-HSO-13`: **Bed Readiness Notification**: Notify bed allocation managers and clinical units when a cleaned bed achieves `READY` status.

7. Clinical Resource Management (`Harmonia-Relevant`)

- **7.1 Specialized Equipment Coordination**
  - `FEAT-HSO-14`: **Mobile Clinical Equipment Tracking**: Track physical location and allocation status of shared clinical equipment (infusion pumps, ventilators, mobile telemetry).

8. Service Capacity Management (`Harmonia-Relevant`)

- **8.1 Facility Demand & Capacity Telemetry**
  - `FEAT-HSO-15`: **Operational Capacity Metric Aggregation**: Aggregate operational capacity telemetry (occupancy rates, wait times, diversion status) across facilities.

11. Mobile Staff Management (`Harmonia-Relevant`)

- **11.1 Mobile Worker Dispatch & Safety**
  - `FEAT-HSO-16`: **Mobile Worker Task Dispatch**: Dispatch community care tasks and urgent visits to roving clinicians' mobile devices.
    - **Architectural scope qualification (2026-10-09 human adjudication)**: Dispatch here concerns integration with external/business mobile work-management mechanisms: exchanging externally established assignment, dispatch and operational progression/status information with applicable worker/context information. Harmonia does not thereby determine or manage clinical work, clinical handover, clinical allocation or EMR worklists. External "work/task" vocabulary does not redefine Harmonia synthetic Task. The Feature name/identifier are retained; the proposed name `MobileWorkTaskManagementIntegration` requires separate human confirmation. See the Business integration boundary.
  - `FEAT-HSO-17`: **Mobile Visit Status Tracking**: Ingest arrival, visit progression, and safety check-in milestones from roving staff.

12. On-Call Management (`Harmonia-Relevant`)

- **12.1 On-Call Directory Resolution**

- FEAT-HSO-18: **Active On-Call Provider Resolution**: Resolve currently active on-call specialists, registrars, and operational managers for a given specialty and hospital at any point in time.

13. Work Allocation & Dispatch (Harmonia-Core)

- **13.1 Operational Work Assignment & Supervision**
  - FEAT-HSO-19: **Work Item Instantiation**: Instantiate operational units of work from clinical triggers (e.g., bed cleaning required, specimen transport required).
  - FEAT-HSO-20: **Worker Matching & Allocation**: Match work items against available staff roles, credentials, and locations based on allocation policies.
  - FEAT-HSO-21: **Work Dispatch Delivery**: Deliver work orders to target workers, devices, or team queues with technical and operational acknowledgement capture.
  - FEAT-HSO-22: **Work Progress Oversight & Escalation**: Monitor work progression against time-to-completion thresholds, escalating stalled or unacknowledged tasks.

16. Clinical Qualification Management (Harmonia-Relevant)

- **16.1 Credential Verification Checkpoint**
  - FEAT-HSO-23: **Operational Credential Check**: Validate that a practitioner possesses active credentials and required competency certs prior to dispatching specialized work.

18. Patient Transport (Harmonia-Core)

- **18.1 Patient Transfer Coordination**
  - FEAT-HSO-24: **Transport Request Ingestion**: Ingest internal (portering) and external (ambulance, inter-hospital) patient transfer requests with clinical mobility requirements.
  - FEAT-HSO-25: **Transport Dispatch & Progress Tracking**: Dispatch transport orders, tracking departure, transit, and destination arrival checkpoints.

19. Clinical Logistics Coordination (Harmonia-Relevant)

- **19.1 Specimen & Supply Courier Tracking**
  - FEAT-HSO-26: **Pathology Specimen Transport Tracking**: Track pickup, custody chain, transport conditions, and laboratory delivery of clinical specimens.

20. Discharge Management (Harmonia-Core)

- **20.1 Discharge Progression & Coordination**
  - FEAT-HSO-27: **Discharge Milestone Tracking**: Track clinical criteria-led discharge milestones (medications dispensed, transportation arranged, home care confirmed).
  - FEAT-HSO-28: **Discharge Readiness & Coordination Oversight**: Track and aggregate multidisciplinary discharge milestones, home care confirmations, and transit readiness across care teams.
  - FEAT-HSO-29: **Discharge Notification Dispatch**: Dispatch departure notifications, updated bed states, and finalized discharge summaries to external care providers upon physical patient exit.

View 5: Intrinsic / Shared Enablement

Intrinsic / Shared Enablement encompasses the horizontal, platform-wide system capabilities required to support all healthcare operating contexts.

RAD source listing (presentation only; no inferred ancestry).

1. Patient Clinical Record [Harmonia-Core]
2. Health Information Exchange [Harmonia-Core]
3. Health Information Access [Harmonia-Core]
4. Health Information Communication [Harmonia-Core]
5. Health Information Control [Harmonia-Core]
6. Clinical Knowledge Services [Harmonia-Relevant]
7. Clinical Collaboration [Harmonia-Core]
8. General Collaboration [Harmonia-Relevant]
9. Workflow & Activity Coordination [Harmonia-Core]
10. Calendar Management [Harmonia-Core]
11. Presentation Services [Harmonia-Core]
12. Information Design Governance [Harmonia-Core]

Critical Domain Distinctions: Intrinsic / Shared Enablement

Health Information Exchange: Four Interaction Models

1. **Submission**: "*I send this to you.*" (Point-to-point push of an authored artifact).
2. **Retrieval**: "*I ask for this.*" (Synchronous or asynchronous pull of specific records).
3. **Distribution**: "*I deliberately send this to identified recipients.*" (Addressed fan-out delivery to designated systems or clinical actors).
4. **Syndication**: "*I publish information/change and subscribers receive matching interests.*" (Publish-subscribe topic/criteria distribution).

Workflow & Activity Coordination: Three Work Units

1. **Work Order**: A unit of work where a **human is doing something** physical or operational (e.g., clean bed 4B, transfer patient to X-ray, draw blood sample).
2. **To Do**: A unit of work where a **human is reviewing, updating, or deciding**, including clinical approvals, sign-offs, and triage reviews.
3. **Task**: A unit of **non-human executable work** (internally a synthetic task) executed by platform automation, daemons, or services. *Execution Semantics*: Workflow Management provides the coordination and state engine through which these three activity types progress.

Clinical Collaboration Context

Clinical Collaboration is **Harmonia-Core** because Harmonia provides **Longitudinal Health Record (LHR)-aware collaboration spaces**. Clinicians can converse within secure multidisciplinary team rooms and query patient context directly (e.g., "*What were Fred's last three HbA1c results?*"). *Important Boundary*: A clinical statement made within a collaboration chat does **not** automatically become an authoritative clinical Observation or medical record entry. It remains collaborative discourse unless formally authored into a clinical document.

Presentation Services Decoupling

Presentation Services consumes established identity, security, and clinical context. Presentation does **not** own:

- Authentication or credential storage.
- Identity lifecycle or EMPI reconciliation.
- Consent decisions or access-policy evaluation.
- Authoritative clinical state preservation.

Detailed Capability & Feature Catalogue: Intrinsic / Shared Enablement

1. Patient Clinical Record (`Harmonia-Core`)

- **1.1 Longitudinal Clinical Record Governance**
  - `FEAT-ISE-01`: **Longitudinal Health Record (LHR) Assembly**: Assemble disparate encounters, diagnostic reports, medications, and clinical documents into a unified, vendor-neutral longitudinal timeline.
  - `FEAT-ISE-02`: **Active Clinical Record Access**: Provide low-latency, active distributed access to consolidated patient clinical summaries.
  - `FEAT-ISE-03`: **Durable Clinical Record Preservation**: Durably maintain governed, vendor-neutral longitudinal clinical representation and preservation with verifiable integrity controls.

2. Health Information Exchange (`Harmonia-Core`)

- **2.1 Multi-Model Information Exchange**
  - `FEAT-ISE-04`: **Direct Submission Ingestion & Delivery**: Support push submissions of clinical documents and messages across boundaries.
  - `FEAT-ISE-05`: **Clinical Record Retrieval Mediation**: Mediate query-based retrieval requests across external and internal repositories.
  - `FEAT-ISE-06`: **Addressed Clinical Distribution**: Fan out clinical events and diagnostic reports to designated external and internal recipients with per-destination delivery tracking.
  - `FEAT-ISE-07`: **Event-Driven Syndication**: Publish clinical and directory change events to subscribed systems matching interest filters.

3. Health Information Access (`Harmonia-Core`)

- **3.1 Governed Query & Search**
  - `FEAT-ISE-08`: **Federated Clinical Query**: Execute bounded, indexed queries across distributed clinical resources and directory graphs.
  - `FEAT-ISE-09`: **Access-Controlled Information Filtering**: Filter search results dynamically according to requesting user authorizations and patient consent directives.

4. Health Information Communication (`Harmonia-Core`)

- **4.1 Boundary Protocol Mediation**
  - `FEAT-ISE-10`: **Standards-Based Boundary Exchange**: Provide standards-conformant boundary exchange for healthcare messaging and resource representations.

5. Health Information Control (`Harmonia-Core`)

- **5.1 Policy & Security Governance**
  - `FEAT-ISE-11`: **Health Information Access & Consent Control**: Evaluate patient consent preferences, practitioner authority boundaries, and access control policies for managed clinical records.
  - `FEAT-ISE-12`: **Security Context Propagation**: Propagate tamper-evident, attributable security context across asynchronous processing boundaries.
  - `FEAT-ISE-13`: **Non-PHI Compliance Auditing**: Generate tamper-evident, verifiable audit records of security-significant events without emitting unmasked PHI.

6. Clinical Knowledge Services (`Harmonia-Relevant`)

- **6.1 Terminology & Reference Mapping**
  - `FEAT-ISE-14`: **Clinical Concept Resolution**: Resolve local clinical codes against standard terminologies (SNOMED CT, LOINC, ICD-10).
  - `FEAT-ISE-15`: **Bidirectional Terminology Mapping**: Translate between legacy local code systems and canonical national healthcare profiles.

7. Clinical Collaboration (`Harmonia-Core`)

- **7.1 LHR-Aware Multidisciplinary Collaboration**
  - `FEAT-ISE-16`: **Care-Team Clinical Collaboration Coordination**: Coordinate secure multidisciplinary clinical communication and consultation exchange bound to patient encounters or clinical topics.
  - `FEAT-ISE-17`: **In-Conversation LHR Query Resolution**: Resolve clinical data queries within an authorized collaboration context, presenting verified clinical summaries directly to care team members.
  - `FEAT-ISE-18`: **Zero-PHI Collaboration Metadata Governance**: Ensure collaboration channel titles, topics, and aliases use opaque identifiers free of patient names, MRNs, or sensitive details.

8. General Collaboration (`Harmonia-Relevant`)

- **8.1 Operational Messaging**
  - `FEAT-ISE-19`: **Operational Team Chat Enablement**: Provide secure instant messaging channels for departmental, administrative, and facility support staff.

9. Workflow & Activity Coordination (`Harmonia-Core`)

- **9.1 Multi-Work-Unit Orchestration**
  - `FEAT-ISE-20`: **Work Order Progression**: Manage execution states and dispatch of human operational work orders.
  - `FEAT-ISE-21`: **To Do Decision & Approval Coordination**: Coordinate human clinical review items, sign-offs, and approval workflows.
  - `FEAT-ISE-22`: **Synthetic Task Orchestration**: Execute non-human, automated platform activity units and sequence orchestration.
  - `FEAT-ISE-23`: **Workflow Timeout & Escalation Oversight**: Supervise operational deadlines, executing automated timeout notifications and supervisory escalations.

10. Calendar Management (`Harmonia-Core`)

- **10.1 Operational & Clinical Calendar Sync**
  - `FEAT-ISE-24`: **Schedule Projection Integration**: Project clinician rosters, on-call schedules, and facility operating calendars into operational views.
  - `FEAT-ISE-25`: **Temporal Event Correlation**: Correlate incoming clinical events with scheduled appointments and operational calendar milestones.

11. Presentation Services (`Harmonia-Core`)

- **11.1 Decoupled Human Interaction & Dashboards**
  - `FEAT-ISE-26`: **Longitudinal Clinical & Operational Presentation**: Deliver contextual clinical timelines, directory administration views, and operational management dashboards.

12. Information Design Governance (`Harmonia-Core`)

- **12.1 Canonical Semantics & Schema Governance**
  - `FEAT-ISE-27`: **Information Standards & Semantic Governance**: Govern canonical clinical data definitions, terminology bindings, and exchange profiles across regional systems.
  - `FEAT-ISE-28`: **Semantic Conformance Verification**: Verify inbound and outbound representations against published semantic rules and conformance constraints.

Health Service Assurance: Approved Business Enabling Capabilities

REQ-FND-005 — Independent Assurance of Governed Activity motivates BC-18 — Health Service Assurance. BC-18 requires the three Business Enabling Capabilities defined here. They are collaboratively realised by the approved contributions of existing Enterprise Capabilities and EC-14 — Service Guardian, as recorded in the contribution matrix.

These are Strategy capabilities and semantic boundaries. Their presentation does not create a sixth contextual view, allocate them to an existing view, establish CT1 / CT2 / CT3 ancestry, or derive Business Functions, Services, Processes, Features or assurance Information Concepts.

Assurance Design

- **Definition**: The capability to determine and define how satisfaction of a governed requirement, constraint or expected behaviour will be assured, including its assurance disposition, applicable criteria, required evidence and evaluation expectations.
- **Fundamental Question**: **How will satisfaction of this requirement, constraint or expected behaviour be assured?**
- **Boundary**: Assurance Design is a peer concern to Behaviour Design and Failure/Recovery Design when implementing governed behaviour. This relationship establishes design concerns, not additional capability entries, application design or implementation specifications.

Assurance Criteria Management

- **Definition**: The capability to establish and manage reusable, governed and temporally identifiable assurance criteria through which subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These are examples, not an exhaustive taxonomy.

- **Boundary**: Criteria management does not own or replace the authoritative governance requirement from which a criterion may derive. Not every assurance criterion is required to derive from formal Governance. Criteria may have their own lifecycle, version and temporal applicability.

  Assurance Criteria may operationalise a governing requirement for evaluation, but SHALL NOT silently alter, weaken, strengthen or replace the authoritative requirement from which they derive.

Governed Assurance

- **Definition**: The capability to independently evaluate a governed subject against applicable assurance criteria through the assembly and evaluation of sufficient trustworthy evidence, establishing explicit findings, exceptions and assurance conclusions.
- **Fundamental Question**: **What can we independently establish about this subject against these criteria from the available trustworthy evidence?**
- **Assurance Semantics**: Establishing the subject/context of assurance, applicable assurance criteria, evidentiary association, assessment, adjudication, findings, exceptions and conclusions.
- **Boundary**: Governed Assurance does not manage, remediate, assign, delegate, escalate operationally or complete the subject activity. It may manage its own assurance activity; findings may inform operational response without transferring responsibility for that response.

Assurance Disposition

Every implemented behaviour has an assurance aspect. Every governed obligation, functional requirement, non-functional requirement or other defined implementation requirement SHALL have an explicit assurance disposition identifying how its satisfaction is assured.

The approved disposition concepts are:

- **Generic Assurance sufficient**
- **Existing Specific Assurance applies**
- **Existing Assurance must be extended**
- **New Specific Assurance required**
- **Not Assurable**
- **Not Worth Assuring**

These are architectural semantics, not an information model or software enumeration. **Not Assurable** does not mean the requirement is not required. **Not Worth Assuring** does not mean the requirement is optional or unimportant. Both are explicit governed assurance decisions, not absence of assurance configuration. An explicit disposition does not require specific assurance of every individual activity, information item or outcome; REQ-FND-005 applies where assurance is required.

Generic Processing Assurance and System Operations

Harmonia-managed workflow behaviour SHALL be subject to applicable generic processing assurance. This concerns business-level processing behaviour above and beyond System Operations. Potential concerns include progression, completion, timeliness, expected processing pathway, failure behaviour, recovery behaviour, outcome, and aggregate or pattern behaviour. This is not a final exhaustive criteria catalogue.

System Operations asks whether the technical machinery is operating. Generic processing assurance asks whether governed business behaviour is operating within expected boundaries.

Operational Health $\neq$ Processing Assurance.

Failure and recovery are part of the expected behavioural space of the activity being executed. The activity/Praxis remains responsible for handling its own expected failure and recovery behaviour. Assurance independently establishes whether normal, failure and recovery behaviour remain within applicable expectations. This responsibility boundary does not allocate assurance execution to Praxis or another solution construct.

EC-12 Operational Assurance retains its established resilience, concurrency, exception/recovery and telemetry responsibilities and may contribute directly to generic operational/processing assurance. EC-14 Service Guardian supplies the distinguishing assurance-specific semantics; neither replaces System Operations or the other Enterprise Capability.

Assurance Evidence Assembly and Information Ownership

Assurance Evidence Assembly is the evidence-assembly concern within Governed Assurance; this statement does not establish a fourth Business Enabling Capability or a downstream Function, Service or Information Concept.

Underlying information used as assurance evidence remains owned and managed by the capability responsible for that information. Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Being evidence is contextual, not an intrinsic universal property of source information.

Evidence may be associated through reference, linkage, aggregation, representation, mirroring or derivation. These possibilities do not prescribe technical storage or replication. An association may identify a particular version, state or temporal context of the underlying information.

> **Evidence inclusion may be temporally and version-specific. Association of information as assurance evidence SHALL be capable of distinguishing the particular information state relevant to the assurance activity from the information's current state.**

Temporally Appropriate Evidence and Criteria

Applicable assurance criteria may themselves be versioned and temporally applicable. A conclusion may depend both on the information state relevant to the evidence and on the criterion state applicable to the subject at the relevant time. Assurance evaluates temporally appropriate evidence against temporally appropriate criteria. No persistence, active-state, replication or retention mechanism is derived here.

Assessment, Adjudication and Explicit Insufficiency

Concern	Fundamental question	Semantic boundary
Assessment	What does this evidence demonstrate with respect to the applicable criterion?	May supply quantitative, qualitative, compound, temporal or confidence-bearing information.
Adjudication	What assurance finding or conclusion follows from that assessment?	Establishes the finding or conclusion from assessed detail; it is distinct from assessment and operational response.

Assurance SHALL NOT be reduced universally to PASS / FAIL / UNKNOWN. Insufficient evidence is not equivalent to either satisfaction or non-satisfaction. Under REQ-FND-005, where available evidence cannot establish the required assurance conclusion with sufficient confidence, that inability and evidence insufficiency SHALL remain explicit.

An assurance activity may execute correctly while lacking sufficient evidence to establish a conclusion about its subject. This is distinct from an indeterminate execution outcome or execution failure. REQ-FND-004 and AX-15 continue to govern uncertainty about operational state, effect or outcome, including the assurance activity's own execution where applicable.

Assurance Independence

Independent Assurance is performed through an independently governed activity whose progression and assurance conclusion are not controlled by the activity or mechanism responsible for performing or managing the subject being assured.

Independence does not require a different organisation, person, application, Kubernetes cluster or database, an external auditor, or physical separation. The subject activity may supply evidence but SHALL NOT solely determine, suppress, manufacture or retrospectively alter its own assurance conclusion.

> **Evidence dependency does not compromise assurance independence; control dependency does.**

Assurance may manage its own assurance activity without assuming management of the subject activity. No technical or organisational separation is prescribed by this boundary.

Clinical Services Delivery Assurance Boundary

Health Service Assurance SHALL NOT be represented as Clinical Services Delivery Assurance. Harmonia does not thereby assume responsibility for assuring clinical judgement, professional practice, clinical adequacy of care, or delivery of clinical services by healthcare practitioners or healthcare organisations.

Information concerning clinical service delivery MAY be the subject of governed assurance where an explicit applicable requirement establishes that concern. This does not transfer responsibility for Clinical Services Delivery Assurance to Harmonia. Capability 06 — Clinical Quality, Safety & Improvement retains its established clinical quality/safety responsibilities.

> **Harmonia may assure facts and behaviour concerning clinical activity where explicitly required; it does not thereby assure the clinical adequacy of care.**

Strategy Derivation Boundary

The approved derivation view stops at collaborative Enterprise Capability realisation. No strategic logical or application component is allocated these capabilities or EC-14. No Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or other implementation relationship is established. Business Architecture, Information Architecture and solution derivation remain subsequent activities.

Downstream Progression

The Business Enabling Capability catalogue and its atomic Features establish what systems must enable for the healthcare enterprise. These features serve as the empirical basis for deriving the reusable, technology-neutral platform capabilities:

- Enterprise Capabilities (EC-01 .. EC-14): The established EC-01 through EC-13 and approved EC-14 Service Guardian; the Health Service Assurance contribution model is established without adding Feature or component allocations.
- Health Service Assurance Derivation: REQ-FND-005 → BC-18 → the three approved Business Enabling Capabilities → collaborative Enterprise Capability realisation.
- Capability Tier Progression Model: Detailed vertical derivation rules and representative composition examples.
- ICT Foundation Lenses: Technical enablement considerations guiding technology realization.

39. Business Architecture Metamodel & Modelling Rules

RAD source: `docs/markdown/03-business-architecture/metamodel/business-architecture-metamodel.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Business Architecture Metamodel & Modelling Rules

1. Overview & Metamodel Hierarchy

The Harmonia Business Architecture metamodel defines the structural and behavioural relationships governing how healthcare capabilities are operationalised into functions, exposed services, stateful processes, and information responsibilities.

RAD diagram: textual source notation.

```
graph TD
    subgraph OWNER["Owning Business / Business Enabling Capability or Feature responsibility context"]
        subgraph BC["Business Capability"]
            BC1["BC CT1"] -->|Decomposes| BC2["BC CT2"]
            BC2 -->|Decomposes| BC3["BC CT3"]
        end
        subgraph BEC["Business Enabling Capability"]
            CT1["BEC CT1"] -->|Capability decomposition| CT2["BEC CT2"]
            CT2 -->|Capability decomposition| CT3["BEC CT3"]
            CT3 -->|Frames every Harmonia Feature| FT["FT Feature<br/>(Capability Specialisation)"]
        end
        BC -.->|Enabled-by / supported-by where established| BEC
    end
    OWNER -->|Delivers| FUNC["Business Function(s)"]
    OWNER -->|Provides to clients| SERV["Business Service(s)"]
    SERV -->|May use directly| FUNC
    SERV -->|May use| PROC
    FUNC -->|May require| PROC
    PROC -->|Chains| FUNC
    OWNER -->|Encompasses where progression is material| PROC["Business Process(es)"]
    OWNER -->|Establishes| INFO["Business Information Responsibility"]
```

The diagram distinguishes two Capability hierarchies and preserves the permissible behavioural responsibility contexts. Business Capabilities describe organisational abilities; Business Enabling Capabilities describe reusable enabling abilities Harmonia provides or supports. Enablement/support is not decomposition or implementation equivalence, and the aggregate diagram establishes no individual BC↔BEC mapping. Business Capability decomposition does not terminate in Harmonia Features. Every fully reconciled Harmonia Feature requires complete **Business Enabling CT1 → CT2 → CT3** ancestry. Functions, Services and Processes may still anchor directly at their established owning Capability or Feature; they do not require an intervening Feature. Service, Function and Process form a graph rather than a mandatory hierarchy. A Function need not have either a Service or a Process.

1.1 Capability Decomposition and Terminology

The following terminology SHALL be used canonically:

Term	Meaning
Layer / Architectural Layer	Architectural layering within the TOGAF / ArchiMate architecture model.

Term	Meaning
Capability Tier	Hierarchical decomposition within a Capability Model.
Derivation / Derivation Progression	Progression between different architectural constructs or models. A Derivation Stage is a stage in that progression.
CT1	Capability Tier 1: establishes the broad Capability frame within Harmonia's architectural scope.
CT2	Capability Tier 2: decomposes the CT1 frame into constituent Capabilities.
CT3	Capability Tier 3: establishes the finest Capability frame required by Harmonia. Within the Business Enabling hierarchy, CT3 provides the context within which Harmonia Features are defined.
FT	Feature: expresses required system-enabled behaviour within its owning Business Enabling CT3 Capability context.

`Layer` ≠ `Capability Tier` ≠ `Derivation Stage`. `L1`, `L2` and `L3` are deprecated as canonical capability classifications.

Harmonia SHALL use the fixed three-tier Capability decomposition:

RAD source listing (presentation only; no inferred ancestry).

Business Capability CT1 → CT2 → CT3

enabled-by / supported-by

Business Enabling Capability CT1 → CT2 → CT3 → FT Feature

Every Harmonia Feature SHALL be immediately framed by a **Business Enabling CT3** Capability. Every established CT3 SHALL have a CT2 parent and every CT2 a CT1 parent within the same Capability category. Capability tiers SHALL NOT be skipped. Complete Business Enabling CT1 → CT2 → CT3 ancestry is required for a fully reconciled Feature; direct CT1 → FT and CT2 → FT framing SHALL NOT be used. Business Capability CT3s do not require or own Harmonia Features. An intentionally undecomposed BC branch does not indicate missing Feature framing.

> **A Feature cannot compensate for incomplete Capability decomposition. If Harmonia requires a Feature, the CT3 Capability that frames that Feature must first be identified. Failure to identify a meaningful CT3 indicates incomplete upstream Capability modelling, not permission to skip the tier.**

Complete decomposition is a modelling obligation, not authority to fabricate architectural elements. CT3 Capabilities SHALL NOT be manufactured merely to satisfy structural symmetry. Where an existing branch contains a Feature without an established CT3 frame or complete ancestry, the Feature, its established responsibility and local identifier SHALL be preserved, and its Capability ancestry/decomposition SHALL be recorded as requiring reconciliation under AX-17. It SHALL NOT be presented as fully reconciled or silently reattached to an invented owner. Established partial relationships remain valid evidence until separately adjudicated.

A Feature remains a finer-grained **Capability Specialisation** within Business Enabling responsibility and inherits Capability semantics. This retained classification does not make FT another Capability Tier, a Business Capability child or an arbitrary-depth frame. CT1 → CT2 and CT2 → CT3 denote same-category Capability decomposition; Business Enabling CT3 provides the mandatory Feature frame. BC↔BEC support relationships do not supply missing ancestry in either hierarchy.

CT3 is Harmonia's intentional Capability decomposition boundary. Harmonia's Capability Model is bounded to the level required to derive and govern Harmonia; it does not attempt to model the complete Enterprise Architecture of a hospital, health service provider, health department or other healthcare enterprise. Harmonia SHALL NOT define CT4, CT5 or deeper Capability tiers. Further detail is not authority to extend the Capability hierarchy.

Established higher-tier ancestry SHALL be represented in a lower-tier Capability's Canonical ID. Every Feature's complete structural Canonical ID SHALL include its CT1, CT2 and CT3 ancestry. An unidentified ancestor SHALL NOT permit omission of that tier; required but unresolved ancestry SHALL leave the Canonical ID unresolved, as specified in §8.

The five-stage Domain02 derivation progression SHALL NOT be reinterpreted as CT1 / CT2 / CT3 / FT. Enablement, derivation and contextual-view membership do not, by themselves, establish a Capability Tier assignment.

The approved fixed-decomposition decision record records the rationale and superseded permissive position. It changes the target metamodel, not the reconciliation status, tier allocation or identity of existing catalogue elements. K9's uncertainty and evidence rules remain controlling.

1.2 Hierarchy Naming & Uniqueness Rules

1. **Global Uniqueness**: Capability names across CT1, CT2 and CT3 are globally unique across the entire Harmonia architecture.
2. **Feature Scoping**: Feature names must be unique within their owning CT1 Capability hierarchy. An unresolved CT1 ancestor SHALL be recorded as unresolved rather than assigned speculatively.
3. **No Disconnected Elements**: Business Functions, Business Services, and Business Processes must **never** be modelled as free-standing global catalogues disconnected from an owning Capability or Feature context.

2. Capability & Feature Semantics

A Capability or Feature establishes:

- **Context & Scope**: Sets the boundary of healthcare operational relevance.
- **Architectural Responsibility**: Establishes unambiguous ownership of behaviour and state.
- **Business Functions**: Defines the internal behaviour delivered within the boundary.
- **Exposed Business Services**: Declares behaviour made available to external or cross-capability consumers.
- **Business Processes**: Encompasses progression lifecycles where activity state tracking is material.
- **Information Responsibility**: Owns the conceptual information created or governed by its functions.
- **Dependencies**: Explicitly records consumption of services provided by other capabilities.

3. Function and Service Semantics

Domain03 SHALL NOT establish a Feature identity that Strategy has not established. A Strategy Feature may be associated with Business behaviour only where that behaviour materially realises the semantic responsibility expressed by the Feature. Shared subject matter, terminology, information, Actor, Capability context or clinical purpose is insufficient. Where semantic realisation is not established, preserve the Feature, Function, Service and established owning Capability, and record **Feature association not established** without assigning a replacement Feature.

The following are **Harmonia architecture definitions**, reconciled by the 2026-10-13 B1 task decision. They apply across architectural views and need not conform to the exact semantics of ArchiMate or another modelling framework. Frameworks may represent them where useful but SHALL NOT redefine their Harmonia meaning. Domain03's owning Capability/Feature and exposure rules continue to qualify Business behaviour; they do not replace these definitions.

3.1 Business Function Rule

> A **Function** is a mechanism that moves the state of information, or the state of a capability concern as a whole, from one status to another. A Function may operate over a small, medium or large concern. Its architectural meaning derives from what it does to or within the capability, not from what it exposes externally.

A Function answers **what change is effected?** Function accountability concerns achievement of the defined state transition. Function size does not determine whether something is a Function.

> **A Business Function represents behaviour delivered within the responsibility boundary of its owning Capability or Feature.**

A Business Function describes *what* the capability does internally to discharge its architectural mandate. A Business Function may operate upon:

- information owned by its Capability / Feature;
- information obtained through Services exposed by another Capability / Feature;
- information used under an explicit cross-capability dependency.

Therefore, **ownership of a Function does not imply ownership of every information item upon which that Function operates**.

3.2 Business Service Exposure Rule

> A **Service** is a mechanism through which a client interacts with a capability to request change, request information, or be informed of change. A Service represents the interface between the capability and its client and operates according to a Service Contract. A Service may use one or more Functions and/or Processes to fulfil that contract.

A Service answers **what can the client ask for, obtain, or be informed about?** Service accountability concerns fulfilment of the **Service Contract**. A Service may use Functions directly, Processes, or both; a Function need not be exposed through a Service.

> **When Function behaviour is exposed for consumption outside the owning Capability or Feature responsibility boundary, that behaviour is exposed through a Business Service.**

RAD source listing (presentation only; no inferred ancestry).

> Capability A (Owner) provides Service X used by Capability B (Client)
>
> fulfilled using
>
> Function(s) and/or Process(es)

3.3 Ownership and Non-Equivalence Invariants

1. **Ownership Preservation**: When Capability A exposes `FunctionX` as `BusinessServiceX`, and Capability B consumes `BusinessServiceX`, **Capability A retains sole architectural ownership of Function X and its underlying state**. Capability B is merely a consumer.
2. **No 1:1 Mechanical Generation**: A Business Service is **not** created mechanically for every Business Function. A Service is modelled *only* when an identifiable consumer exists outside the owning responsibility boundary.
3. **Consumption Scope**:
   - **Harmonia-Internal Consumption**: Services consumed by other Harmonia capabilities (e.g., Service Administration consuming Person Identifier Resolution from Entity Management).
   - **Enterprise/External Consumption**: Services consumed by external healthcare participants, client systems, or national registries (e.g., Referral Submission exposed to external GP practices).

3.4 Significance-Driven Relationships and Typed Dependencies

Role, Actor, Function, Service, Interaction, Collaboration and Process relationships SHALL be established where they materially explain responsibility, participation, exposure, collaboration

or Business progression. Their absence alone does not establish missing architecture. Exhaustive Function-to-Role, Function-to-Service, Interaction-to-Service or Actor-to-Function matrices are not required.

A Service dependency requires consuming behaviour that needs behaviour actually exposed by that Service. Related information or subject matter is insufficient. Capability dependency, Service consumption, Interaction participation and Collaboration involvement SHALL retain their distinct types and meanings. Descriptive or abbreviated references do not create aliases. An unestablished element or relationship SHALL remain explicitly unresolved; no replacement dependency is inferred.

3.5 Clinical and Operational Privilege

Clinical Privilege concerns whether a practitioner is authorised/credentialed to undertake applicable clinical activity. **Operational Privilege** concerns whether an actor is authorised to undertake applicable operational activity. They are distinct responsibilities; an activity may require both.

Practitioner Role information may contextualise or contribute to a privilege determination, but Practitioner Role Resolution establishes neither Clinical Privilege nor Operational Privilege. These distinctions do not allocate new Roles, Services or policy mechanisms.

3.6 Business Responsibility and Reusable Composition

Business Architecture SHALL describe responsibility meaningful to the Business at its declared architectural layer. Shared downstream machinery does not establish a shared Business Function. In service-delivery operational contexts, **Capacity Management** concerns understanding and managing available, committed and utilised service capacity. Contexts may have materially different capacity semantics. A generalised operational-state capture mechanism is a downstream derivation concern and SHALL NOT replace those Business responsibilities. The HSO capacity responsibility boundary distinguishes context-specific capacity from resource state and higher-order consumers.

A domain-specific Business Enabling Feature MAY be realised through composition of established reusable capabilities and configured workflow/Praxis without a correspondingly specialised Business Function. The composition must sufficiently express the required responsibility; clinical purpose or setting alone does not establish another Harmonia information-management responsibility. Do not create a Feature-to-Function relationship where only a composition-level sufficiency decision is established.

3.7 Clinical Work Integration Boundary

Harmonia may integrate and coordinate around clinical work; it does not thereby manage clinical work. It may exchange work information, receive externally established requirements, route operational activity, observe progression, coordinate Harmonia-managed consequences and apply established qualification, privilege and policy constraints. It SHALL NOT thereby determine clinical work, manage clinical handover, acquire clinical allocation or decision authority, own clinical worklists or assume EMR workflow ownership. Information assembly for handover is distinct from performing or managing the handover.

Clinical facts retain their originating authority when consumed by patient-flow, capacity or discharge coordination. Clinical progression and operational patient progression SHALL remain distinct. A clinical/professional clearance may affect discharge readiness without the operational consumer acquiring authority to establish that clearance. Clinical Privilege and Operational Privilege retain §3.5's distinct meanings.

Harmonia terminology remains **Work Order = human doing**, **To Do = human review/update/approval**, and **Task = synthetic task**. External work/task vocabulary does not redefine Harmonia Task. Coordination of a To Do supports an authorised review or approval; it does not confer clinical-work management authority.

4. Business Process Semantics

> A **Process** is an ordered chain of Functions used to realise a complex change. A Process provides a convenient mechanism for decomposing, structuring and managing change that would otherwise be difficult to understand or control as a single Function. A Function may itself require one or more Processes where the change for which that Function is responsible requires further decomposition.

A Process answers **how is a complex change decomposed and coordinated?** Process accountability concerns progression and coordination of its constituent Functions. Function and Process may be recursively composed:

RAD source listing (presentation only; no inferred ancestry).

> Function
>
> may require → Process
>
> Function
>
> Function
>
> Function
>
> may require → Process

A Business Process applies this meaning to progressing Business behaviour within an owning Capability or Feature responsibility boundary. There is no mandatory Service → Process → Function hierarchy.

4.1 Process Justification Rule

> **Model a Business Process where ordered Function decomposition is material to understanding, structuring or managing a complex Business change and its progression.**

Meaningful states, dispositions and outcomes may justify that decomposition. Not all Functions require a Process. Search, lookup, mapping, verification or presentation labels alone neither establish a Process nor exclude one where the bounded change requires decomposition. Primary Information traceability remains with the responsible Functions; Process-level aggregation does not replace it. Existing named Business Processes and responsibility boundaries are preserved; this rule creates no new catalogue entry.

4.2 Cross-Capability Process Guardrail

If a proposed Business Process appears to span multiple Capability responsibility boundaries:

1. Examine whether the process has been scoped incorrectly (conflating independent sequential activities with a single process); or
2. Examine whether an architectural responsibility is missing or misassigned.

Cross-capability processes must never be used to erase or blur Capability ownership boundaries. Cross-boundary coordination is achieved via event notifications and service invocations between bounded capabilities.

4.3 Behaviour and Process Consistency

A Business Process may elaborate progression for a defined activity within its owning Capability or Feature without reproducing the Behaviour stage list. It SHALL preserve established responsibility, ownership, authority boundaries and supported obligations. Scoped applicability, compound checkpoints and alternative dispositions SHALL remain explicit; additional Process detail SHALL NOT silently become a universal requirement or redefine owning responsibility.

Omission of a Process checkpoint from a Behaviour summary does not invalidate that checkpoint; omission of Behaviour responsibility from a Process does not remove that responsibility. Stage-count equality is not required. Greater Process detail does not override established responsibility.

4.4 Illustrative Progression and Material Outcome Uncertainty

Illustrative progression SHALL NOT imply an authoritative transition model. Only established states/transitions may be normative. Where alternatives, terminal behaviour or post-completion relationships remain unresolved, both the surrounding text and the affected diagram SHALL visibly identify the uncertainty.

Where Harmonia manages or governs an activity with a materially significant outcome and available evidence cannot establish that outcome, the uncertainty SHALL remain explicit. Absence of acknowledgement, response, observation or evidence does not by itself establish success or failure. This follows REQ-FND-004 and AX-15; it creates no universal INDETERMINATE Process state or downstream execution-state machinery.

5. Cross-Cutting Responsibility Rule

Harmonia establishes an essential architectural guardrail to maintain component cohesion and prevent boundary erosion:

> **Cross-cutting concern does not imply cross-cutting responsibility. A concern becomes cross-cutting because Capabilities depend upon Services or Functions delivered by other Capabilities whose responsibilities remain explicitly bounded.**

Concrete Guardrails:

- **Security & Control**: Security does not become partly owned by every capability that consumes security services. Authorisation evaluation and security context propagation are owned exclusively by *Health Information Control*.
- **Semantic Governance**: Terminology and canonical model definitions do not become owned by capabilities that use them. They are owned by *Information Design Governance* and *Clinical Knowledge Services*.
- **Workflow & Activity Coordination**: Coordinating tasks and to-dos does not grant ownership of the underlying clinical or operational activities. Coordination semantics are owned by *Workflow & Activity Coordination*.
- **Presentation**: Presentation capabilities do not acquire ownership of the clinical information they render.
- **Information Exchange**: Transporting or mediating messages across networks does not grant ownership of the clinical payloads transported. Exchange state is owned by *Health Information Exchange*, while clinical document ownership remains with *Clinical Record Administration*.

6. Business Information Responsibility

> **Business information ownership derives strictly from Capability responsibility and the Functions/Processes through which that responsibility is discharged.**

- **No Ownership Transfer**: Access, use, transformation, transport, search, presentation, caching, persistence, or coordination of information does not transfer architectural ownership.

- **Pure Business Semantics**: Business information concepts must retain authentic clinical/business definitions. They must **never** be prematurely translated into:
  - FHIR Resource structures (e.g., `Patient`, `Observation`, `Task`);
  - Database schemas, tables, or DDL;
  - JSON schemas, DTOs, or wire payloads;
  - Java classes or programming interfaces.

Domain04 formalises conceptual information meaning, semantic relationships and responsibility traceability. Realised representations are downstream: application logical/software representation in Domain05; exchange contracts, interoperability profiles, wire payloads and transformations in Domain06; physical platform/runtime/product/deployment realisation in Domain07. Persistence realisation may involve Domain05 and Domain07; a more precise allocation is not established. Representation does not define upstream business or conceptual meaning.

6.1 Management, Originating Authority and Historical Truth

Managing, preserving, governing access to, communicating, contextualising or maintaining the history of information SHALL NOT imply originating authority for the represented facts.

Within Harmonia's operational boundaries, Harmonia may maintain durable, immutable, append-only historical truth concerning what it received, knew, asserted, managed, decided, communicated or did. Subsequent correction, supersession or changed knowledge SHALL NOT retrospectively alter that historical record. Current authoritative state and historical operational truth remain distinct. This does not make every transient observation durable evidence or weaken the active/durable-state separation.

6.2 Business Terminology and Context

Interpret an element according to its declared architectural layer and Business meaning. Cache, index, routing, topic, session and similar terms do not prescribe a technology or product merely because those words also have technical uses. Preserve clear Business meanings; implementation structures or bindings require explicit downstream authority.

Where Healthcare Service context is material to an activity's meaning, authority, coordination, progression or accountability, that context SHALL remain identifiable through relevant Business behaviour and information responsibility. Organisation, Location, Practitioner or Role association does not imply that context. The obligation is contextual, not universal, and defines no representation, identifier structure, persistence mechanism, FHIR representation or implementation binding.

7. Qualified Architectural Reference Grammar

To unambiguously express participants, their acting capacity, and situational context without inventing transient metamodel entities, Harmonia uses the **Qualified Architectural Reference Grammar**:

RAD source listing (presentation only; no inferred ancestry).

```
<EntityType>#<Entity>-as-<Role>[#<ContextQualifier>]
```

7.1 Grammar Structure

- `<EntityType>`: A rendering of the canonical Business Actor category (`Person`, `Group`, `Organisation`, `OrganisationalUnit`, `Government/RegulatoryBody`, `System`, `Device`).
- `<Entity>`: The specific business instance identifier or name.
- `<Role>`: The canonical Business Role representing the stable business/functional capacity being fulfilled.

- `[#<ContextQualifier>]`: *(Optional)* The contextual function or situational qualifier in a specific interaction or collaboration.

Qualified references resolve to canonical architectural identities. For compact rendering, CamelCase SHOULD be used where possible: Information Supplier → `InformationSupplier`, Service Provider → `ServiceProvider`, Organisational Unit → `OrganisationalUnit`. This is syntactic rendering, not an alias or independent element. Preserve meaning and disambiguate where a compact expression could resolve to more than one identity; compact spelling alone SHALL NOT establish identity.

7.2 Canonical Examples

Qualified Expression	Actor Category	Entity Name	Fulfilled Role	Context Qualifier
`Organisation#ACTPathology-as-InformationSupplier`	`Organisation`	ACT Pathology	`InformationSupplier`	*(none)*
`Organisation#ACTHealth-as-ServiceProvider#ReferralSource`	`Organisation`	ACT Health	`ServiceProvider`	`ReferralSource`
`System#ACTPathologyLIS-as-InformationSupplier#DiagnosticResultSource`	`System`	ACT Pathology LIS	`InformationSupplier`	`DiagnosticResultSource`
`Person#Fred-as-Patient`	`Person`	Fred	`Patient`	*(none)*
`Person#DrSmith-as-Clinician#Reviewer`	`Person`	Dr Smith	`Clinician`	`Reviewer`
`OrganisationalUnit#FinanceDirectorate-as-InformationCustodian`	`OrganisationalUnit`	Finance Directorate	`InformationCustodian`	*(none)*
`GovernmentRegulatoryBody#AHPRA-as-Regulator`	`Government/RegulatoryBody`	AHPRA	`Regulator`	*(none)*

7.3 Governance Guardrails

1. The **Role** identifies stable architectural/business capacity.
2. The **Context Qualifier** identifies situational nuance; it does **not** create a new Role.
3. This notation is a **reference grammar**, not a new architectural metamodel class.

8. Architectural Element Identity and Canonical Identification

8.1 Common Identity Properties

Every governed Business Architecture element SHALL have the following identity properties:

Property	Cardinality	Meaning
Element Type	Exactly one	The element's architectural semantic type.
Canonical Name	Exactly one	The current authoritative architectural name.
Canonical ID	Exactly one current value	The Canonical Architectural Identifier expressing current architectural responsibility context.

Property	Cardinality	Meaning
Name Alias	Zero or more	A retained alternative or historical name for the same element.
ID Alias	Zero or more	A retained non-canonical identifier resolving directly to the same current element.

These properties SHALL preserve the semantic distinctions between Capability, Feature, Business Function, Business Service and Business Process. Existing canonical-name uniqueness and ownership rules remain applicable.

This convention establishes the identification requirement; it does not assert that the existing architecture corpus already conforms. Where required ancestry or identifier allocation is not established, the Canonical ID SHALL remain explicitly unresolved. A placeholder or speculative identifier SHALL NOT be treated as an allocated Canonical ID.

8.2 Typed Canonical Identifier Grammar

The general form is `<architectural-context>.<element-identifier>`. A root CT1 Capability has no preceding context. For Capability, Feature, Business Function, Business Service and Business Process elements, the structural grammar is:

RAD source listing (presentation only; no inferred ancestry).

```
canonical-id =
capability-path
| feature-path
| capability-path "." behaviour-token
| feature-path "." behaviour-token
capability-path = ct1-token [ "." ct2-token [ "." ct3-token ] ]
ct3-path = ct1-token "." ct2-token "." ct3-token
feature-path = ct3-path "." feature-token
ct1-token = "CT1-" <approved-local-code>
ct2-token = "CT2-" <approved-local-code>
ct3-token = "CT3-" <approved-local-code>
feature-token = "FEAT-" <approved-feature-code>
behaviour-token =
"FN-" <approved-local-code>
| "SV-" <approved-local-code>
| "PR-" <approved-local-code>
```

Codes SHALL be approved, nonempty tokens without the context separator `.`. This grammar establishes no code allocation or numeric width. Existing established Feature identifiers SHALL retain their canonical spelling, including `FEAT-EM-01`; `FEAT-EM01` SHALL NOT be substituted or assumed equivalent.

Capability tokens SHALL explicitly carry their Capability Tier type. Untyped forms such as `CT01`, `CT02` and `CT03` SHALL NOT be used as canonical Capability tokens. The FN / SV / PR prefixes identify behaviour types and SHALL NOT constitute additional Capability Tiers.

The following are syntax examples only; they SHALL NOT assign identifiers or Capability Tiers to existing architectural elements:

RAD source listing (presentation only; no inferred ancestry).

CT1-01

CT1-01.CT2-02

CT1-01.CT2-02.CT3-03

CT1-01.CT2-02.CT3-03.FEAT-EM-01

CT1-01.FN-01

CT1-01.CT2-02.PR-01

CT1-01.CT2-02.CT3-03.SV-01

CT1-01.CT2-02.CT3-03.FEAT-EM-01.FN-01

Every ancestor prefix in a current Canonical ID SHALL identify the corresponding current Capability or Feature in the established responsibility ancestry. `CT1-01.CT3-02` SHALL NOT be generated merely because CT2 ancestry has not been identified. Unresolved ancestry SHALL remain unresolved rather than being omitted or manufactured.

The shorter `capability-path` forms identify CT1 or CT2 Capabilities and permit direct behavioural anchoring; they SHALL NOT frame a Feature token. A Feature, or behaviour anchored beneath it, requires `feature-path` with complete Business Enabling CT1 → CT2 → CT3 ancestry. A syntactically valid path under a Business Capability does not establish valid Harmonia Feature ownership. These are grammar rules only and allocate no identifier to an existing element.

The common identity properties apply to other governed Business Architecture element types, but this decision does not establish their token vocabularies or structural contexts. Those SHALL NOT be inferred from this grammar. The Qualified Architectural Reference Grammar in §7 remains distinct and unchanged.

8.3 Behavioural Anchoring and Contextual Containment

Business Functions, Business Services and Business Processes MAY be anchored directly at any Capability Tier or Feature establishing responsibility for that behaviour. CT2, CT3 or FT elements SHALL NOT be invented merely to provide behavioural anchoring.

Canonical identifier containment SHALL express architectural responsibility context. It SHALL NOT redefine semantic relationships or Element Type, and SHALL NOT be interpreted as implementation containment. A Function beneath CT2 remains a Function; a Service beneath CT1 remains a Service; a Process beneath a Feature remains a Process. Function delivery, Service exposure and justified Process progression retain the rules in §§2–4.

The complete Canonical ID SHALL preserve contextual architectural standing in flat documents, generated diagrams, models, traceability matrices, cross-document references and AI contexts without depending on Markdown nesting. A flat representation retaining that context is permitted; disconnected ownership is not.

8.4 Rename, Relocation and Alias Rules

A Canonical Name change alone SHALL NOT change architectural identity or require a Canonical ID change. A previous name MAY be retained as a Name Alias for historical or search purposes.

The approved rename `Clinical Credential Management → Clinical Qualification Management` preserves the existing practitioner professional / clinical qualification and competency responsibility. `ClinicalCredentialManagement` MAY be retained as a Name Alias only for that same element. It SHALL remain distinct from Domain03 `CredentialManagement`, whose separately established responsibility concerns operational badges and physical access. The rename SHALL NOT establish equivalence or aliasing between those elements.

Where an element changes architectural responsibility context, its Canonical ID SHALL change to express current structural truth. The previous Canonical ID SHALL be retained as an ID Alias. This rule also applies to descendant elements whose responsibility paths change. The following relocation is a syntax example only:

RAD source listing (presentation only; no inferred ancestry).

Previous Canonical ID: CT1-01.CT2-02.FN-01

Current Canonical ID: CT1-01.CT2-02.CT3-03.FN-01

ID Alias: CT1-01.CT2-02.FN-01

An ID Alias SHALL:

- refer to the same architectural element;
- be searchable and resolvable;
- be non-canonical;
- resolve directly to the current architectural element, without requiring an alias chain;
- never be used for a new authoritative architectural reference.

Canonical IDs and ID Aliases SHALL occupy one globally unique, non-reusable identifier namespace. Once an identifier has referred to an element, canonically or as a historical Canonical ID, it SHALL NOT subsequently identify another element. An ID Alias SHALL NOT collide with a current Canonical ID or an ID Alias assigned to another element. All retained aliases SHALL continue resolving directly after subsequent relocations. New authoritative references SHALL use the current Canonical ID.

Name Aliases are names, not members of the identifier namespace. Neither Name nor ID Aliases SHALL establish equivalence merely from similar wording or nested behaviour. The approved G1 K5 rules remain controlling: finer-grained Functions or Services SHALL NOT be promoted to Features, or assigned a broader Feature's identity, solely to obtain traceability symmetry.

8.5 Machine-Validation and Generation Intent

This convention is intended to support future automated architectural navigation, diagram generation, model generation, traceability matrices, cross-document references and AI-assisted architecture interpretation. It SHALL permit validation of at least:

- Canonical ID uniqueness;
- ID Alias uniqueness;
- Canonical ID / ID Alias collisions;
- ancestor existence and complete, correctly typed ancestry;
- valid element-type suffix or terminal token;
- valid Capability / Feature responsibility context;
- resolvable references;
- direct, unambiguous alias resolution;
- orphaned identifiers.

This decision implements no validators, schemas or classes. Generation intent does not require the exhaustive capability matrices excluded by Domain02's modelling position.

8.6 Approved K9 Derivation and Uncertainty Rules

Approved G1 K9 found no affected legacy owner assignment sufficiently validated to authorise mechanical conversion to CT1 / CT2 / CT3. Established identities and responsibilities remain valid; affected Capability Tier, complete ancestry and root status remain unresolved.

Unresolved Capability Tier or ancestry SHALL NOT prevent downstream derivation from an otherwise established owning Capability or Feature and its relevant responsibility. Downstream traceability SHALL reference that element without manufacturing structural ancestry. Document structure, catalogue grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity.

`Client Administration → Person Identity → Identifier Resolution [FT / FEAT-EM-01]` is an established partial responsibility chain and SHALL be preserved. Its full structural Canonical ID remains unresolved until actual Capability Tier ancestry and identifier allocation are architecturally established. The former incorrect L3 example SHALL NOT supply that ancestry.

The fixed model in §1.1 does not retrospectively validate legacy L1 / L2 / L3 assignments or convert a known owner into CT3. Existing Features with incomplete ancestry remain valid architectural evidence requiring Capability decomposition reconciliation; downstream derivation from their established responsibility does not declare them structurally reconciled. Missing CT3 frames SHALL be separately established before subsequent Feature definition, without using catalogue presentation to manufacture them.

These rules preserve AX-01/AX-04 business meaning, AX-14 distinctions, AX-15 uncertainty and AX-17 architectural authority and explicit uncertainty. They allocate no structural IDs, manufacture no ancestry and do not authorise Package2 Information Family derivation.

The subsequent R1/R2 Business Capability reconciliation explicitly establishes its stated BC tiers/parents independently of catalogue indentation. It does not resolve any affected historical BEC ancestry, convert any of the 69 enabling descriptions into Capabilities, frame the 137 Features or allocate Canonical IDs. K9 continues to govern all unestablished dimensions.

9. R1.x/R2.x Semantic Sufficiency Boundary

The human architectural adjudication of 2026-10-09 establishes semantic sufficiency rather than graph density. Domain03 must express the Business behaviour required by Strategy without mirroring Strategy's structure. Complete identifier allocation is not a prerequisite for semantic completeness; deterministic corrections preserve established identities, while unresolved ancestry, tiers and code allocations remain explicit.

Semantic completeness requires sufficient Business expression to derive Strategy's required behaviour. It requires neither one Function per Feature, one Service per Function, exhaustive Role/Function or Feature/Function mappings, identical decomposition nor structural symmetry. A composition/workflow realisation under §3.6 may suffice without a specialised Function; unestablished individual relationships remain unestablished. The residual SD-03, SD-08 and SD-14 decisions are recorded in the Service Delivery catalogue, while HSO-09 and HSO-16 are reconciled within their operational responsibilities.

The current approved assurance authority boundary and the three assurance Process purposes/progression descriptions are sufficient for this baseline. Detailed approval allocation and lifecycle transitions remain unestablished and may be revisited if later architecture demonstrates a genuine Business requirement. Evidentiary use alone establishes no Service, Interaction or dependency. The approved assurance catalogue remains two Roles, five Functions, three Processes, three Services and three Interactions; financial governance exclusion and non-recursion remain intact.

The approved G1 capability-scoped corrections and unresolved Feature/dependency relationships remain valid unless a separately authorised semantic decision establishes more. No exhaustive Value Stream derivation, participant/exposure matrix, Twin Business Actor or workflow, or organisational risk-management responsibility is required for structural completeness.

40. Harmonia Capability Decomposition & Derivation Progression

RAD source: `docs/markdown/02-strategy/capability-maps/capability-tier-model.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Harmonia Capability Decomposition & Derivation Progression

Overview

This model distinguishes hierarchical Capability decomposition from the derivation progression, composition relationships, and derivation rules connecting high-level healthcare enterprise capabilities with reusable platform capabilities and downstream architectural responsibility boundaries.

Canonical terminology SHALL distinguish:

- **Layer / Architectural Layer**: architectural layering within the TOGAF / ArchiMate architecture model.
- **Capability Tier**: hierarchical decomposition within a Capability Model.
- **Derivation / Derivation Progression**: progression between different architectural constructs or models; a Derivation Stage is a stage in that progression.

`Layer ≠ Capability Tier ≠ Derivation Stage.`

Business Capability decomposition SHALL use `CT1 → CT2 → CT3`; Business Enabling Capability decomposition SHALL use `CT1 → CT2 → CT3 → FT`. BC↔BEC enabled-by/support is a separate relationship, not decomposition or implementation equivalence. In these fixed models: `CT1` means Capability Tier 1, `CT2` Capability Tier 2, `CT3` Capability Tier 3 and `FT` Feature. `L1`, `L2` and `L3` remain deprecated as canonical capability classifications.

Tier / type	Architectural role
CT1	Establishes the broad Capability frame within Harmonia's architectural scope.
CT2	Decomposes the CT1 frame into constituent Capabilities.
CT3	Establishes the finest Capability frame required by Harmonia and within the Business Enabling hierarchy, the Capability context within which Harmonia Features are defined.
FT	Expresses required system-enabled behaviour within its owning Business Enabling CT3 Capability context.

Every Harmonia Feature SHALL be immediately framed by Business Enabling CT3; Business Capabilities do not decompose into Harmonia Features; every CT3 SHALL have a CT2 parent; every CT2 SHALL have a CT1 parent. Capability tiers SHALL NOT be skipped. Complete CT1 → CT2 → CT3 ancestry is required for a fully reconciled Feature. CT1 → CT2 and CT2 → CT3 are Capability decomposition relationships.

> **A Feature cannot compensate for incomplete Capability decomposition. If Harmonia requires a Feature, the CT3 Capability that frames that Feature must first be identified. Failure to identify a meaningful CT3 indicates incomplete upstream Capability modelling, not permission to skip the tier.**

This modelling obligation SHALL NOT authorise manufactured CT3 Capabilities for structural symmetry. Existing Features without established CT3 frames SHALL be preserved with their

established responsibilities and local IDs; their Capability ancestry/decomposition requires reconciliation. Missing ancestry SHALL remain explicitly unresolved, rather than being omitted, invented or treated as direct CT1/CT2 Feature framing in the target model.

CT3 is the intentional Capability decomposition boundary. Harmonia models the level required to derive and govern Harmonia, rather than the complete Enterprise Architecture of a hospital, health service provider, health department or other healthcare enterprise. CT4, CT5 and deeper Capability tiers are outside that scope and SHALL NOT be defined. The approved decision record records the rationale and supersession; the Domain03 metamodel governs the fixed hierarchy.

The Domain03 Business Architecture metamodel governs typed Canonical IDs, identity, aliases and behavioural anchoring. The five-stage derivation progression below SHALL NOT be reinterpreted as CT1 / CT2 / CT3 / FT or used to assign Capability Tiers or identifier codes to existing elements. Approved G1 K9 establishes that affected legacy owner assignments are insufficiently evidenced; they SHALL NOT be mechanically translated to CT1 / CT2 / CT3. Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved where not established.

Unresolved Capability Tier or ancestry SHALL NOT prevent downstream derivation where the owning Capability or Feature and its relevant responsibility are established. Traceability SHALL reference that element without manufacturing ancestry, as required by AX-17. Document structure, grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity.

The model ensures that:

1. Enterprise healthcare practice is clearly demarcated from software enablement.
2. System-enabled capabilities reflect authentic clinical, administrative, and operational contexts rather than integration software mechanics.
3. System behaviours are decomposed into atomic, testable features.
4. Reusable capabilities (EC-01 .. EC-14) are derived systematically without creating centralized bottleneck services.
5. Technical realisations are guided by ICT Foundation lenses without allowing physical software products to define architectural capabilities.

1. The Five-Stage Derivation Progression

The framework progresses through five derivation stages relating different architectural constructs and models. Their stage numbers SHALL NOT denote Capability Tiers or Architectural Layers:

RAD source listing (presentation only; no inferred ancestry).

STAGE 1: BUSINESS CAPABILITY

What the enterprise must be capable of doing

(18 catalogued capabilities across 5 natural healthcare regions)

requires enablement by

▼

STAGE 2: BUSINESS ENABLING CAPABILITY

What systems must enable or provide in support of the business

(Structured across 5 authentic healthcare contextual views)

specifies atomic behaviour as

▼

STAGE 3: FEATURE (FT)

Smallest useful statement of required system-enabled behaviour

(Independently understandable, testable, technology-neutral)

derived into reusable

▼

STAGE 4: ENTERPRISE CAPABILITY (EC-01 .. EC-14)

Reusable ICT functionality recurring across multiple domains

(Decoupled from concurrency, message brokers, and storage DDL)

viewed through

▼

STAGE 5: ICT FOUNDATION CAPABILITY LENSES (18 Lenses)

Cross-cutting technical enablement considerations

(Evaluates technical realization via technology-substitution test)

realised by

▼

ARCHITECTURE REALISATION

Domains 03–07: Business, Information, Application, Integration, Tech

2. Derivation Stage Definitions

Stage 1: Business Capability

- **Definition**: What the healthcare enterprise must be capable of doing to deliver healthcare services, govern operations, and ensure patient safety.
- **Independence**: Exists independently of IT systems, software products, commercial organizational hierarchies, or automation.
- **Structure**: 18 CT1 Business Capability roots grouped into five natural regions, with the explicit CT2/CT3 assertions and deliberate stopping points in the canonical BC catalogue. Their tiers are established by that reconciliation, not derivation-stage position; Canonical IDs remain unallocated:
  - *Care & Health Delivery* (01–08)
  - *Health Information & Digital Health* (09–12)
  - *Research & Innovation* (13)
  - *Health Service Management* (14–16)
  - *Governance & Assurance* (17–18)
- **Harmonia Principle**: Harmonia provides core technical enablement for Health Information and Connected Health capabilities (09–12) and materially enables delivery and workforce capabilities (01, 02, 04, 06, 15) without asserting platform ownership over healthcare business practice.

Natural regions are contextual groupings, not capability tiers, organisational structures, application boundaries or ownership hierarchies, and are distinct from Business Enabling contextual views. Health Service Governance (17) retains unresolved relevance classification/enabling derivation; Health Service Assurance (18) is Harmonia-Relevant under the reconciled BC catalogue, preserving its approved independent governed-assurance participation. The approved Health Service Assurance derivation establishes only REQ-FND-005 → BC-18 → the three named Business Enabling Capabilities → collaborative Enterprise Capability realisation. Stage diagrams and existing examples do not establish additional relationships.

Stage 2: Business Enabling Capability

- **Definition**: What software systems and information infrastructure must enable or provide to support the enterprise healthcare capabilities.
- **Structuring Axis**: Organized into five contextual views reflecting genuine healthcare operating environments:

1. *Entity Management*: Governance of practitioners, organizations, locations, services, products, and devices.
2. *Service Administration*: Administrative progression of encounters, referrals, orders, scheduling, and billing.
3. *Service Delivery*: Systems enablement across clinical delivery settings (primary, acute, emergency, diagnostic, virtual).
4. *Health Service Operations*: Healthcare facility logistics, bed management, work allocation, dispatch, and discharge coordination.
5. *Intrinsic / Shared Enablement*: Longitudinal clinical records, health information exchange, clinical collaboration, and workflow coordination.

- **Independence**: Independent of commercial software product boundaries (PAS, EMR, LIS, RIS) and internal middleware engines.

The approved Assurance Design, Assurance Criteria Management and Governed Assurance additionally support BC-18. Their contextual-view placement, relevance classification, Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved. Their direct Strategy derivation does not create intermediate Features, a sixth view or inferred hierarchy.

Stage 3: Feature (FT)

- **Definition**: The **smallest useful statement of required system-enabled behaviour** within its owning CT3 Business Enabling Capability context. Existing owners whose tier/ancestry is unresolved remain established responsibility evidence requiring reconciliation. FT is a Feature classification; derivation stage 3 SHALL NOT classify it as CT3.
- **Criteria**:
  - *Independently understandable*: Conveys a self-contained operational meaning.
  - *Testable in principle*: Allows clear verification that the behavior occurred or was enabled.
  - *Technology-independent*: Contains no product names, protocols, or storage mechanisms.
  - *Component-independent*: Does not prescribe which software module executes it.
- **Scope Demarcation**: Formally authored for Harmonia-Relevant and Harmonia-Core capabilities in the established catalogue. Reference and Adjacent capabilities are intentionally not decomposed into features. This scope statement allocates no Capability Tier to existing elements and does not waive the fixed ancestry required for a fully reconciled Feature.

Stage 4: Enterprise Capability (EC-01 .. EC-14)

- **Definition**: Reusable architectural functionality derived from recurring system-enabled features across multiple healthcare domains.
- **Derivation Logic**: Derived by analyzing features and asking: "*How is this function delivered?*" Functions that recur vertically across multiple distinct healthcare domains are abstracted into reusable Enterprise Capabilities.
- **Catalogue**: The established EC-01 Managed Entity & Relationship through EC-13 Semantic Governance & Conformance, plus approved EC-14 Service Guardian.
- **Collaborative Nature**: Enterprise Capabilities represent bounded reusable responsibilities. Their realisation may require participation, consumption or enforcement across multiple architectural elements, but this does not imply shared semantic ownership or prohibit establishment of a responsibility centre. **Distributed participation does not imply distributed responsibility. Cross-cutting concern does not imply cross-cutting responsibility.**

EC-14 supplies the assurance-specific semantics needed to bind existing reusable capabilities into governed assurance. The approved first-pass contribution matrix establishes collaborative realisation without extending EC-12's semantics or allocating EC-14 to a component. The generic progression through Features describes the existing derivation convention; it does not manufacture Feature relationships for this explicitly approved assurance derivation.

Stage 5: ICT Foundation Capability Lenses

- **Definition**: Technical enablement lenses that provide cross-cutting engineering criteria, architectural considerations, and technology-substitution testing for realizing Enterprise Capabilities.
- **Role**: They are technical lenses, NOT peers of the Business Enabling capabilities.
- **Test**: Governed by the *Technology-Substitution Test*: "*If the named product, protocol, programming language, or deployment mechanism were replaced tomorrow, would the capability still exist?*"

3. The Capability Composition Formula

Harmonia capabilities are composed according to the following foundational relationship:

Harmonia Capability = domain-specific enabling behaviour + reusable Enterprise Capabilities

Where:

- **Domain-Specific Enabling Behaviour**: The specific clinical or operational logic, context rules, and progression semantics unique to a healthcare domain (e.g., healthcare identifier system rules, diagnostic order workflows, bed turnover readiness).
- **Reusable Enterprise Capabilities**: The horizontal, platform-wide capabilities (EC-01 through EC-14) that provide state management, context propagation, policy evaluation, provenance capture, interoperable transport and the approved assurance-specific semantics.

Informing Strategic Logical Component Boundaries

In Harmonia Strategy:

- **Capabilities describe *what* the platform achieves**.
- **Strategic Logical Components describe *how responsibility is organized***.

When specific combinations of Enterprise Capabilities and domain-specific behaviours recur coherently across multiple features, they inform candidate **Strategic Logical Component responsibility boundaries**:

RAD source listing (presentation only; no inferred ancestry).

RECURRING COMPOSITION PATTERN

Domain-specific enabling behaviour

+ Reusable Enterprise Capabilities

informs clustering for

▼

STRATEGIC LOGICAL RESPONSIBILITY MODEL

- Mneme (Managed Information & Active State)
- Mnemosyne (Durable Preservation & Recovery)
- Ponos (Managed Operational Activity Execution)
- Pylai (Standards-Conformant Ingress/Egress Membrane)
- Calliope (Semantic Authority & Conformance)
- Iris (Contextual Human Interaction & Presentation)
- Digital Twin (Entity Coordination Archetype)

Architectural Principle: This composition informs component boundaries; it does **not** imply a rigid, mathematical one-to-one derivation. Components represent cohesive responsibility centres, not one-to-one wrappings of individual capabilities.

The displayed component-clustering examples remain the established EC-01 through EC-13 derivations. This convention does not allocate EC-14 or the three assurance Business Enabling Capabilities to a strategic logical component or establish a relationship to an existing implementation construct.

4. Representative Derivation Examples

To demonstrate how Enterprise Capabilities recur across distinct Business Enabling features without constructing an exhaustive matrix, three representative derivation examples are detailed below.

Example 1: Person Identifier Resolution

In healthcare integration, identifying an individual across disparate clinics, hospitals, and diagnostic centres requires resolving identifiers (MRNs, national healthcare identifiers, local practice IDs) issued by multiple independent authorities.

- **Domain-Specific Enabling Behaviour**:
  - Healthcare identifier system semantics (e.g., verifying issuing authority jurisdiction, assigning authority namespaces).
  - Scope of identifier validity (temporary vs. permanent, national vs. regional).
  - Client privacy and consent flags associated with identifier disclosure.
- **Reusable Enterprise Capability Contributions**:
  - **EC-01 Managed Entity & Relationship**: Modeling the Person entity and linking multiple scoped identifiers and alias relationships.
  - **EC-05 Search & Discovery**: Locating existing entity records matching supplied identifier sets and resolving candidate matches.
  - **EC-03 Managed State & Lifecycle**: Governing the lifecycle status (active, superseded, suspended) of identifier records.
  - **EC-07 Provenance & Traceability**: Recording the authoritative origin, verification timestamp, and issuing system attribution for each identifier.
  - **EC-06 Policy & Control**: Enforcing default-deny access policies governing which requesting systems may view or resolve specific identifier authorities.
  - **EC-12 Operational Assurance**: Ensuring deterministic matching behavior and transactional consistency during concurrent resolution requests.

Example 2: Order Closed Loop

A clinical diagnostic or procedure order requires governed progression from initial request to final diagnostic report.

- **Domain Progression Semantics**: Destination → Route → Deliver → Acknowledge → Progress → Complete → Outcome
- **Critical Healthcare Distinction**: Technical Delivery Acknowledgement $\neq$ Business Acknowledgement A technical acknowledgement establishes only the acceptance state defined by the applicable interaction contract. It SHALL NOT be interpreted as durable acceptance, committed business state, successful processing or successful business disposition unless that meaning is explicitly established by the governing contract. HTTP 200/202 and MLLP acknowledgements are interpreted under their applicable contracts. A handoff acknowledgement alone does not establish the eventual downstream business outcome or the receiving laboratory's clinical responsibility. REQ-FND-001 retains its explicit Harmonia durable-ingress requirement.

- **Reusable Enterprise Capability Contributions**:
  - **EC-02 Context Management**: Establishing and propagating the clinical encounter, ordering provider, and subject-of-care context across all order lifecycle events.
  - **EC-05 Search & Discovery**: Resolving the target fulfillment service endpoint and receiving facility capabilities.
  - **EC-01 Managed Entity & Relationship**: Associating the Order entity with the Subject, Ordering Practitioner, Performing Organization, and Target Service.
  - **EC-08 Interoperability & Exchange**: Standards-compliant boundary protocol adaptation, message transformation, delivery dispatch, and transport acknowledgement capture.
  - **EC-03 Managed State & Lifecycle**: Tracking the formal Order state progression (`PLACED` → `RECEIVED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - **EC-10 Activity & Execution**: Instantiating and executing operational units of work, tracking timeouts, managing escalations, and supervising fulfillment deadlines.
  - **EC-07 Provenance & Traceability**: Capturing complete chain-of-custody, transformation history, and delivery timestamps for clinical auditability.
  - **EC-12 Operational Assurance**: Guaranteeing dual-write safety, duplicate suppression, and exception recovery across asynchronous boundary handoffs.

Example 3: Work Allocation & Dispatch

Operational healthcare activities (e.g., patient transport, ward bed preparation, discharge coordination, emergency triage assessment) require assigning and dispatching work to qualified clinical or operational staff.

- **Domain Progression Semantics**: Event → Entity Context → Required Activity → Assignment → Dispatch → ACK → Progress → Outcome
- **Healthcare Service Context Rule**: Where operational activity occurs in the context of a Healthcare Service, that service context must be explicitly associated with the activity/state rather than inferred solely from practitioner, organization, or location.
- **Cross-Domain Recurrence**: This operational progression pattern recurs across:
  - *Discharge Progress*: Coordinating multidisciplinary discharge milestones and readiness oversight.
  - *Patient Transport*: Allocating transfer tasks to portering staff based on mobility requirements.
  - *Mobile Work*: Dispatching community health visits to roving practitioners.
  - *ED Operational Coordination*: Progressing triage queue assessments.
  - *Bed Readiness*: Dispatching environmental cleaning staff upon patient discharge.
- **Reusable Enterprise Capability Contributions**:
  - **EC-02 Context Management**: Binding the patient, physical bed/room, care team, and healthcare service context to the operational work item.
  - **EC-03 Managed State & Lifecycle**: Managing the lifecycle of the operational activity (`REQUESTED` → `ASSIGNED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - **EC-05 Search & Discovery**: Discovering available, rostered, and credentialed practitioners or operational personnel matching task criteria.
  - **EC-01 Managed Entity & Relationship**: Linking the assigned worker, patient, location, and equipment entities.
  - **EC-10 Activity & Execution**: Governing the assignment logic, dispatch mechanism, human notification, timeout monitoring, and escalation triggers.
  - **EC-08 Interoperability & Exchange**: Delivering work orders to mobile devices or external task management systems where external dispatch is required.
  - **EC-12 Operational Assurance**: Preventing duplicate assignment and ensuring resilient recovery if a dispatched worker does not acknowledge within defined thresholds.
  - **EC-07 Provenance & Traceability**: Recording assignment timestamps, worker acceptance, and completion evidence.

5. Architectural Modelling Position: Omission of Exhaustive N × M Matrices

Harmonia explicitly rejects the generation of exhaustive N × M traceability matrices between Business Capabilities and Business Enabling Capabilities, or between Features and Enterprise Capabilities.

Formal Rationale

> **The Business Enabling Capability model was derived with explicit consideration of the Business Capability model and is intended to provide the system-enabled capabilities necessary to support that business landscape. A formal many-to-many mapping between Business Capabilities and Business Enabling Capabilities has not been produced, as such traceability is not required for the current Harmonia architecture and strategy objectives. Such mapping may be developed subsequently where required for benefits realisation, investment analysis, business-case development or other value-traceability purposes.**

This statement records the existing derivation. BC-17 enabling derivation and relationships beyond the approved BC-18 derivation remain unresolved under AX-17. The Health Service Assurance view adds only the approved three Business Enabling Capabilities and bounded fourteen-row EC contribution matrix. That explicit matrix is not an exhaustive Business Capability-to-Business Enabling or Feature-to-Enterprise mapping, does not imply other coverage, and does not change EC-12's established operational semantics.

Utilitarian Justification

1. **Avoidance of False Precision**: In a complex regional HIE, a single Business Capability (e.g., *Care Access & Coordination*) draws upon dozens of enabling capabilities across Entity Management, Service Administration, and Shared Enablement. An exhaustive matrix suggests mechanical one-to-one connections where real systems operate through dynamic, contextual composition.
2. **Maintenance Overhead vs. Value**: Maintaining a static matrix of hundreds of capabilities across thousands of cells creates substantial documentation drift without providing actionable guidance to software engineers or architects.
3. **Clarity of Representative Derivations**: As shown in Section 4, representative derivation patterns provide clear, verifiable architectural rationale for how enterprise needs compose reusable capabilities, without the clutter of exhaustive combinatorial tables.

Topic Navigation

- Domain 02 Strategy Overview
- Capability Framework Index
- Business Capabilities
- Business Enabling Capabilities
- Enterprise Capabilities
- Health Service Assurance Strategy Derivation
- ICT Foundation Lenses

41. Health Service Assurance — Approved Strategy Derivation

RAD source: `docs/markdown/02-strategy/capability-maps/health-service-assurance-derivation.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Source qualification: The opening 2026-10-14 reconciliation qualifies the older unresolved BC-18 tier/relevance statements later in this source. Later approved Domain03 assurance behaviour is reproduced separately; earlier Strategy stop boundaries retain their own scope.

Health Service Assurance — Approved Strategy Derivation

> **BC reconciliation — 2026-10-14:** The current BC catalogue establishes BC-18 as a CT1, its stated BC CT2/CT3 decomposition and Harmonia-Relevant classification. Its macro-level confidence purpose preserves REQ-FND-005's applicable independent governed-assurance requirement. This qualifies the earlier unresolved BC-tier/relevance statements below; all existing BEC/EC support assertions remain unchanged and are not redistributed among the new BC descendants. BEC tiers, ancestry and identity allocations remain unresolved.

Purpose and Architectural Authority

This view records the approved Health Service Assurance derivation in Domain02 Strategy. It connects the existing Motivation requirement and Business Capability to three approved Business Enabling Capabilities and their collaborative Enterprise Capability realisation. Canonical definitions remain in the linked capability catalogues.

AX-17 — Architectural Authority and Explicit Uncertainty governs truthful traceability: only the relationships explicitly approved here are established. Missing relationships remain unresolved. The requested Strategy reconciliation is approved architectural input; it does not complete later architecture domains.

The materially relevant axioms also include:

- AX-14 — Semantic Distinctions Are Preserved: assurance is distinct from monitoring, evidence production, operational management, clinical adequacy, assessment and adjudication.
- AX-06 — Information Authority Is Explicit, AX-07 — Security Is Intrinsic to Managed Operations and AX-08 — Evidence Records Meaning, Not Machinery: trustworthy evidence and governed assurance preserve their existing authority, security and meaningful-evidence constraints without transferring source-information ownership.
- AX-09 — Transient Operational State Is Ephemeral by Default: contextual evidence association does not automatically make all observations or operational diagnostics durable evidence.
- AX-10 — Distribution, Load and Failure Are Normal Operating Conditions and AX-15 — Uncertainty Is Preserved Until Resolved: expected failure/recovery remains the subject activity's responsibility, and operational uncertainty remains distinct from insufficient assurance evidence.
- AX-05 — Active State and Authoritative Durable State Are Distinct: temporal/version-specific evidence does not alter the approved state-responsibility separation or derive a persistence/active-state implementation.

Motivation to Strategy Traceability

RAD diagram: textual source notation.

```
graph TD
    REQ["REQ-FND-005<br/>Independent Assurance of Governed Activity"] --> BC["BC-18<br/>Health Service
    ↪  Assurance"]
    BC --> AD["Business Enabling Capability<br/>Assurance Design"]
    BC --> ACM["Business Enabling Capability<br/>Assurance Criteria Management"]
    BC --> GA["Business Enabling Capability<br/>Governed Assurance"]
    AD --> COL["Collaborative Enterprise Capability realisation<br/>Approved contribution matrix"]
    ACM --> COL
    GA --> COL
    COL --> EXIST["Applicable existing EC-01 through EC-13<br/>Direct or Supporting as explicitly approved"]
    COL --> SG["EC-14<br/>Service Guardian"]
```

The final branches identify contributions to collaborative realisation, not ownership, execution order or component allocation. The matrix below distinguishes the contribution to each Business Enabling Capability; the aggregate diagram does not assert every existing EC contributes to each capability.

Established relationship	Authoritative Strategy reference	Boundary
REQ-FND-005 → BC-18	Approved requirement; Health Service Assurance	Independent conclusion authority and explicit evidence insufficiency; no clinical-adequacy responsibility transfer.
BC-18 → Assurance Design	Assurance Design	Defines how satisfaction is assured, not application design.
BC-18 → Assurance Criteria Management	Assurance Criteria Management	Manages criteria without replacing governing requirements.
BC-18 → Governed Assurance	Governed Assurance	Independently evaluates and concludes without managing the subject.
Three Business Enabling Capabilities → collaborative EC realisation	Contribution matrix; Enterprise catalogue	Direct does not mean sole realisation; no extra relationships implied.
EC-14 → assurance-specific contribution	Service Guardian	Reusable assurance semantics, distinct from EC-12; component allocation remains unresolved.

BC-18 refers to the existing Business Capability catalogue entry **18 Health Service Assurance**. This reference does not assign a new Capability Tier or structural Canonical ID. The three Business Enabling capability names identify the approved responsibilities without inventing CT1 / CT2 / CT3 ancestry or new Feature identifiers.

Collaborative Enterprise Capability Contribution Matrix

This is the approved **first-pass contribution model**. Existing canonical EC names are preserved. **Direct** identifies a direct contribution, not sole realisation or ownership. **Supporting** identifies a supporting contribution. **No Material Role** records the approved absence of a material contribution in this model; it is not an invitation to invent a relationship.

Enterprise Capability	Assurance Design	Assurance Criteria Management	Governed Assurance
EC-01 Managed Entity & Relationship	Supporting	No Material Role	Supporting
EC-02 Context Management	Supporting	Supporting	Direct
EC-03 Managed State & Lifecycle	Supporting	Direct	Direct
EC-04 Information Management	Direct	Direct	Direct

Enterprise Capability	Assurance Design	Assurance Criteria Management	Governed Assurance
EC-05 Search & Discovery	Supporting	Supporting	Supporting
EC-06 Policy & Control	Direct	Direct	Supporting
EC-07 Provenance & Traceability	Supporting	Supporting	Direct
EC-08 Interoperability & Exchange	No Material Role	Supporting	Supporting
EC-09 Event & Subscription	Supporting	No Material Role	Direct
EC-10 Activity & Execution	Supporting	No Material Role	Direct
EC-11 Interaction & Experience	Supporting	Supporting	Supporting
EC-12 Operational Assurance	Supporting	Supporting	Direct, limited to generic operational/ processing assurance contribution
EC-13 Semantic Governance & Conformance	Supporting	Direct	Supporting
EC-14 Service Guardian	Direct	Direct	Direct

The three Business Enabling Capabilities are collaboratively realised. EC-14 contributes the assurance-specific semantics not truthfully represented by EC-01 through EC-13. Their contributions do not transfer source-information ownership, governing-requirement authority or subject-activity management to assurance. In particular, EC-10's direct contribution does not make Governed Assurance responsible for managing the subject activity, and it does not establish a relationship to an execution component.

This bounded, approved matrix is not an exhaustive Business Capability-to-Business Enabling or Feature-to-Enterprise mapping. It is consistent with the existing representative-derivation convention and does not manufacture missing traceability.

Preserved Semantic Boundaries

- **EC-12 / EC-14**: EC-12's established operational responsibilities remain intact. Its direct Governed Assurance contribution is limited to generic operational/processing assurance. EC-14 supplies assurance disposition, criteria applicability, evidentiary association, assessment, adjudication and independent findings/conclusions; EC-12 is not expanded to absorb them.
- **Clinical assurance**: Health Service Assurance is distinct from Clinical Services Delivery Assurance. Explicitly required assurance of facts or behaviour concerning clinical activity does not make Harmonia responsible for clinical judgement, professional practice or clinical adequacy of care. Capability 06 retains its established responsibility.
- **Evidence and time**: Source-information ownership and contextual evidence association remain distinct. Evidence and criteria may each be temporal/version-specific without deriving storage, replication or persistence mechanisms.
- **Assurance and operations**: Generic processing assurance evaluates business behaviour, including expected failure/recovery; the subject activity manages its own behaviour. Operational Health $\neq$ Processing Assurance.
- **Assessment and conclusion**: Assessment, adjudication and explicit insufficiency preserve quantitative, qualitative, compound, temporal and confidence-bearing meaning. Insufficient evidence establishes neither satisfaction nor non-satisfaction.
- **Independence**: Assurance independence concerns control of progression and conclusion, not physical separation. Evidence dependency does not compromise assurance independence; control dependency does.

Unresolved Relationships and Downstream Boundary

This approved derivation establishes only REQ-FND-005 → BC-18 → the three named Business Enabling Capabilities → the first-pass collaborative EC contribution model, including EC-14. The five-region/eighteen Business Capability model, BC-17 Health Service Governance, capabilities 06 and 14, the existing thirteen EC semantics and approved AX-05 reconciliation remain intact.

The following remain deliberately unresolved or unestablished:

- Relevance classification for BC-17/BC-18; BC-17 enabling derivation; relationships beyond the approved BC-18 derivation.
- Contextual-view placement, relevance classification, Capability Tier, complete ancestry, root status and structural Canonical IDs for the three assurance Business Enabling Capabilities; additional Feature decomposition.
- Courses of Action, Strategic Resources, Value Stream/stage relationships and additional capability contributions not explicitly approved here.
- Downstream Business Functions, Services, Processes, actors, roles, assurance Information Concepts, outcome taxonomies, criterion catalogues and applicability/confidence models beyond the approved Strategy semantics.
- Strategic logical component, application component, execution, persistence, protocol, deployment and implementation allocation for EC-14 and the three assurance Business Enabling Capabilities.

No relationship to Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or another implementation construct is derived. Existing Dokimasia material is not reconciled or modified. Domain03 Business Architecture and Domain04 Information Architecture are unchanged. Existing or future suggestions cannot establish an allocation through lexical similarity, familiar implementation or the conceptual behavioural sequence of Service Guardian.

Navigation

- Domain02 Strategy
- Capability Framework
- Business Capability Catalogue
- Business Enabling Capability Catalogue and Assurance Principles
- Enterprise Capability Catalogue
- Capability Derivation Progression
- Capability Maps Index

42. B2.1 — Individual Care Delivery ↔ Health Information Access

RAD source: `docs/markdown/05-information-systems-architecture/traceability/b2-1-individual-care-delivery-health-information-access.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

B2.1 — Individual Care Delivery ↔ Health Information Access

B2.1 — Individual Care Delivery ↔ Health Information Access: PASS

1. Authority and bounded result

This record reconciles the explicitly agreed Domain05 Step B B2.1 decisions, 2026-10-13. The Domain02 Business Capability catalogue defines Individual Care Delivery and its six L2 capabilities. The established Health Information Access responsibility defines the enabling anchor. The Step B method governs relationship semantics and Health Information Access scope.

PASS applies only to the L1 relationship and six L2 relationships below. It is not a runtime verification, a declaration of exhaustive capability coverage or an application/component allocation. This record establishes no additional capability decomposition, Feature, Service, Function or Information mapping. **B2.2 is not commenced.** No other Business Capability or Business Enabling Capability is mapped by this reconciliation.

2. L1 relationship and rationale

`IndividualCareDeliveryenabled-byHealthInformationAccess`

Individual Care Delivery requires governed access to relevant longitudinal and current health information to support clinical assessment, diagnosis, treatment decisions, interventions and monitoring. Health Information Access enables access to that information but does not determine its clinical significance or perform the clinical decision-making of Individual Care Delivery.

3. L2 relationships and rationales

These are material capability-enablement relationships. They do not transfer clinical accountability or require each runtime exercise to retrieve information through Health Information Access. The L2 labels identify the agreed Domain02 decomposition; no additional structural Canonical ID scheme is established.

01.01 Individual Health Assessment

`IndividualHealthAssessmentenabled-byHealthInformationAccess`

Individual Health Assessment requires governed access to relevant existing and current health information so that available clinical, health and contextual information can be considered when establishing an understanding of the individual's health state. Health Information Access enables retrieval of that information but does not perform the assessment or establish the assessed findings.

Health Information Access does not own every mechanism by which assessment information is acquired. Direct examination, observation, history-taking or measurement may establish assessment information independently.

01.02 Clinical Determination

`ClinicalDeterminationenabled-byHealthInformationAccess`

Clinical Determination requires governed access to relevant current and historical health information where that information is necessary to interpret assessed findings and establish clinically meaningful conclusions about the individual's health state, condition, risk or prognosis. Health Information Access enables retrieval of that information but does not interpret its clinical significance or establish the clinical determination.

01.03 Individual Care Planning

`IndividualCarePlanningenabled-byHealthInformationAccess`

Individual Care Planning requires governed access to relevant current and historical health and care information where that information is necessary to establish an appropriate intended clinical response, including care objectives, interventions and monitoring. Health Information Access enables retrieval of that information but does not establish the care intent, select the clinical response or perform the planning.

Information concerning operational availability of services, resources, staff, facilities or appointments is not Health Information Access merely because it influences care planning.

01.04 Clinical Intervention

`ClinicalInterventionenabled-byHealthInformationAccess`

Clinical Intervention requires governed access to relevant health and care information where that information is necessary to safely and appropriately perform an intended clinical action, including information concerning the individual's current health state, established care intent and clinically relevant history. Health Information Access enables retrieval of that information but does not authorise, select or perform the clinical intervention.

Access to information describing an intended intervention does not confer authority to perform that intervention.

01.05 Individual Health Monitoring

`IndividualHealthMonitoringenabled-byHealthInformationAccess`

Individual Health Monitoring requires governed access to relevant current and historical health information where that information is necessary to establish change, stability or deviation in an individual's health state over time. Health Information Access enables access to prior, baseline and current health information required for comparison, but does not perform the monitoring or interpret the clinical significance of an identified change.

01.06 Individual Care Review

`IndividualCareReviewenabled-byHealthInformationAccess`

Individual Care Review requires governed access to relevant health and care information in order to evaluate delivered care and observed health progression against the intended care objectives. Health Information Access enables access to the care intent, relevant care history and health outcomes required for that evaluation, but does not perform the review or establish the continuing appropriateness of the care intent.

4. Preserved boundaries

The Domain02 decomposition and context rules apply: the six established children are now explicitly reconciled as BC CT2s. The current catalogue establishes the stated assessment, monitoring and review CT3s; the other branches retain their recorded uncertainty/stopping points. Under the fixed Capability model, subsequently required Harmonia Features must first have meaningful **Business Enabling CT3** frames established. BC decomposition does not terminate in Features and this B2.1 support trace establishes no ancestry or new CT3 enablement edge. None may be invented for structural completeness. Retained L1/L2 display labels alone do not allocate CT tiers. Examples are not subordinate Capabilities, and care setting, delivery mode, temporal pattern and organisational arrangement do not themselves define different business abilities. Acute, Planned/Scheduled and Longitudinal Care are not the ICD L2 decomposition.

B2-R07 distinguishes individual clinical / health / care information from operational / workforce / financial / corporate / service-resource information. No exhaustive edge-case taxonomy or allocation to other enabling capabilities is introduced.

`enabled-by` does not mean that Health Information Access interprets, judges, plans, authorises or performs clinical care. It expresses material enablement, not a mandatory runtime dependency. B2.1 PASS does not change retained candidate deferrals, declare B1 PASS or authorise subsequent Step B mappings.

43. Principal Business Processes

RAD source: `docs/markdown/03-business-architecture/processes/business-processes.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Principal Business Processes

1. Process Demarcation & Justification Rules

The named owning Capabilities retain their established responsibilities. Affected Capability Tier, complete ancestry, root status and full structural Canonical IDs remain unresolved under approved G1 K9. The explicit `Client Administration → Person Identity` relationship remains a partial chain, not a tier allocation.

Processes may elaborate scoped progression without reproducing Behaviour stage lists. Established ownership, responsibility, authority boundaries and supported obligations remain controlling. Omitted Process checkpoints are not invalidated by a Behaviour summary; omitted Behaviour responsibilities are not removed by a Process. Stage-count equality is not required, and extra detail does not establish universal applicability.

Illustrations are not complete normative transition models. Their scoped applicability and visible uncertainty annotations retain independent Business meaning; detailed execution-state machinery remains downstream. For a materially significant managed outcome that available evidence cannot establish, preserve uncertainty rather than infer success or failure from an absent acknowledgement, response or observation. No universal additional Process state is prescribed.

Where Healthcare Service context is material to the activity's meaning, authority, coordination, progression or accountability, retain that context in the relevant activity/information responsibility. Location, Organisation, Practitioner or Role cannot substitute for it; no representation or implementation binding follows.

In the Harmonia Business Architecture, a **Business Process** applies the canonical Process definition: ordered Function decomposition of complex change within the owning Capability or Feature responsibility. Existing activity-instance states, dispositions and outcomes retain their established meaning; no new transition model is inferred.

1.1 Process Inclusion Rule

> **Model a Business Process where ordered Function decomposition is material to understanding, structuring or managing a complex Business change and its progression.**

A Function may require further Processes recursively. Search, query, resolution, translation, mapping, verification, lookup and presentation do not automatically require a Process, but their labels do not exclude decomposition where justified. Service fulfilment may use Functions directly, Processes, or both. Function-level Information relationships remain primary; this reconciliation adds no named Process to the catalogue.

1.2 Canonical Business Process Catalogue

The sixteen pre-existing Business Processes remain in the current R1.x/R2.x baseline with the authorised identity/discharge reconciliations and explicit lifecycle qualifications. Three approved Health Service Assurance Processes extend this catalogue to **nineteen Business Processes**; their existing authority/progression boundaries are sufficient for this baseline while detailed lifecycle states and transitions remain unestablished.

RAD source listing (presentation only; no inferred ancestry).

Principal Business Processes

Entity & Administrative Processes

1. Governed Person Identity Correction
2. Practitioner Verification

Clinical Lifecycle & Service Administration Processes

3. Referral Progression
4. Encounter Lifecycle
5. Closed-Loop Order Progression
6. Clinical Document Lifecycle

Healthcare Facility & Logistics Operations Processes

7. Theatre Case Progression
8. Clinic / Operational Queue Progression
9. Bed Turnover
10. Operational Work Progression
11. Patient Transport
12. Specimen Transport
13. Discharge Progression

Horizontal Workflow Coordination Processes

14. Work Order Progression (Human Doing)
15. To Do Progression (Human Reviewing / Deciding)
16. Synthetic Task Progression (Automated System Work)

Health Service Assurance Processes

17. Assurance Process Design
18. Assurance Process Execution
19. Assurance Process Reporting / Communication

2. Entity & Administrative Processes

2.1 Governed Person Identity Correction Process

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Process Purpose**: Processes externally authoritative identity corrections and merge/unlink outcomes, applies their consequences to Harmonia-managed identity information and identifier associations, and communicates resulting changes with an audit history.
- **Authority boundary**: Person correction/merge decisions originate externally. Internal triage, evidence verification and approval concern applicability and governed processing of that authoritative outcome, not person matching, golden-record determination, master-person selection or originating merge adjudication.

- **State Progression Lifecycle**: Correction Requested → HIM Triage → Evidence Verified → Correction Approved → Merge/Unlink Executed → Change Broadcasted → Closed
- **Key State Dispositions**:
  - `CorrectionRequested`: Externally authoritative correction or merge/unlink outcome received for governed application.
  - `HIMTriage`: Health Information Manager reviews the received decision/outcome and its applicability.
  - `EvidenceVerified`: Authoritative source decision and supporting provenance/documentation verified.
  - `CorrectionApproved`: Governed application of the externally authoritative outcome approved; this does not originate a person-merge decision.
  - `Merge/UnlinkExecuted`: External merge/unlink consequences applied to Harmonia's managed identifier associations and audit history.
  - `ChangeBroadcasted`: Downstream clinical systems notified of merged or rectified identity.

2.2 Practitioner Verification Process

- **Owning Capability**: `ProviderAdministration`
- **Process Purpose**: Governs the lifecycle of validating, certifying, and periodically re-verifying a healthcare practitioner's professional credentials, registration status, and clinical scope of practice.
- **State Progression Lifecycle**: Verification Initiated → Regulatory Registry Queried → Credentials Validated → Privileges Endorsed → Active Verified → Expired / Suspended
- **Key State Dispositions**:
  - `VerificationInitiated`: New practitioner registration or periodic re-credentialing triggered.
  - `RegulatoryRegistryQueried`: Real-time query executed against national registry (e.g., AHPRA).
  - `CredentialsValidated`: Primary qualifications, specialist endorsements, and sanctions checked.
  - `PrivilegesEndorsed`: Health service medical administration endorses admitting/procedural scope.
  - `ActiveVerified`: Practitioner verified for clinical ordering and electronic communication.
  - `Expired/Suspended`: Verification lapses or regulatory sanction triggers privilege suspension.

3. Clinical Lifecycle & Service Administration Processes

3.3 Referral Progression Process

- **Owning Capability**: `ReferralAdministration`
- **Process Purpose**: Governs the end-to-end operational progression of an incoming clinical referral from receipt through specialist clinical triage, booking, and final service acceptance.
- **Scope qualification**: This illustrates a specialist booking/attendance pathway; Referral does not universally require acceptance, scheduling, attendance or discharge, or establish delivery, responsibility actually assumed or Transfer of Care. Decline, rejection and redirection remain valid dispositions without invented transition paths. `Accepted/Waitlisted` is a local compound checkpoint, not universal equivalence between acceptance and waitlisting.
- **State Progression Lifecycle**: Submitted → Intake Validated → Clinically Triaged → Accepted / Waitlisted → Scheduled → Consultation Attended → Discharged / Rejected (Illustrative specialist pathway; dispositions vary)
- **Key State Dispositions**:
  - `Submitted`: Electronic referral received from GP or external facility.
  - `IntakeValidated`: Administrative validation of patient details, mandatory fields, and tests.
  - `ClinicallyTriaged`: Senior clinician assigns urgency category (e.g., Cat 1 urgent within 30 days).
  - `Accepted/Waitlisted`: Referral accepted and placed onto the specialty waitlist.

- `Scheduled`: Outpatient appointment allocated and patient notified.
- `ConsultationAttended`: Patient seen; initial specialist assessment completed.
- `Discharged/Rejected`: Referral completed and discharged back to primary care, or formally rejected with clinical rationale.

3.4 Encounter Lifecycle Process

- **Owning Capability**: `Episode&EncounterAdministration`
- **Process Purpose**: Governs the clinical and administrative lifecycle of an acute, emergency, inpatient, or outpatient encounter between a patient and healthcare services.
- **State Progression Lifecycle**: Planned / Booked → Arrived → Triaged / Ingested → Active In-Progress → Discharged → Completed / Encoded (Scoped illustration; ON_LEAVE relationship unresolved)
- **Key State Dispositions**:
  - `Planned/Booked`: Elective admission or clinic appointment scheduled.
  - `Arrived`: Patient presents at facility or emergency desk.
  - `Triaged/Ingested`: Clinical triage category assigned; encounter record activated.
  - `ActiveIn-Progress`: Inpatient care, bedside monitoring, and clinical orders actively underway.
  - `Discharged`: Patient clinically discharged; care-place vacated.
  - `Completed/Encoded`: Clinical documentation finalized, ICD/DRG coding completed, and encounter legally closed.
- **Scope qualification**: These completion criteria retain their locally described applicability; they do not establish universal signing, legal closure or originating authority over clinical information. Finalisation, signing and legal qualification remain distinct. The relationship with Strategy `ON_LEAVE`, including applicability and transitions, remains unresolved.

3.5 Closed-Loop Order Progression Process

- **Owning Capability**: `OrderAdministration`
- **Process Purpose**: Governs the rigorous, closed-loop tracking of diagnostic pathology, radiology, and procedural orders from requisition to result correlation, preventing dropped or unfulfilled investigations.
- **Scope qualification**: These examples describe a diagnostic/procedural progression and do not exclude medication from the broader Order Administration responsibility. Specimen and result checkpoints are scoped. The signed requisition/report requirements remain within this illustrated scope; signing is not universally equivalent to finalisation or authority. Result consumption/binding does not define originating validity, authority, authorship or legal status.
- **State Progression Lifecycle**: Requisition Placed → Order Dispatched → Specimen Collected / Scheduled → In-Execution → Preliminary Result Bound → Final Result Bound → Closed / Verified (Scoped diagnostic/procedural illustration)
- **Key State Dispositions**:
  - `RequisitionPlaced`: Electronic order created and signed by requesting clinician.
  - `OrderDispatched`: Order routed and accepted by performing diagnostic service. This local compound checkpoint retains routing and recipient acceptance as distinct facts; dispatch alone establishes neither receipt nor acceptance. Behaviour `Received` is not equated with this checkpoint without evidence.
  - `SpecimenCollected/Scheduled`: Bio-specimen collected or imaging appointment booked.
  - `In-Execution`: Laboratory analysis or diagnostic scanning underway.
  - `PreliminaryResultBound`: Critical or interim observations published and linked to order.
  - `FinalResultBound`: Authoritative diagnostic report signed and bound to order.
  - `Closed/Verified`: Requesting clinician acknowledges result; order closed. This Order-closure checkpoint does not establish diagnostic-content verification, approval or clinical incorporation.

3.6 Clinical Document Lifecycle Process

- **Owning Capability**: `ClinicalRecordAdministration`
- **Process Purpose**: Governs the versioning, clinical sign-off, addenda, superseding, and legal status of clinical documents (discharge summaries, specialist letters, advance care directives).
- **State Progression Lifecycle**: Draft → Preliminary → Final Signed → Amended / Addended → Superseded → Entered-in-Error (Scoped illustration; no universal document lifecycle)
- **Key State Dispositions**:
  - `Draft`: Incomplete clinical document saved during consultation.
  - `Preliminary`: Document authored pending senior registrar or consultant countersignature.
  - `FinalSigned`: Authoritative clinical document legally signed and published.
  - `Amended/Addended`: Governed supplementary clinical addendum appended to signed document.
  - `Superseded`: Entire document replaced by a newer revision; prior version retained for audit.
  - `Entered-in-Error`: Document formally retracted due to wrong patient or invalid content; content struck through with retraction notice.
- **Scope qualification**: `Final` $\neq$ `automatically Final Signed`. The described signing/countersigning and legal qualifications remain local requirements, not a universal document lifecycle. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Amendment, supersession and entered-in-error are not mandatory stages for every document, and consumer processing does not establish originating authority or legal status.

4. Healthcare Facility & Logistics Operations Processes

4.7 Theatre Case Progression Process

- **Owning Capability**: `TheatreOperations`
- **Process Purpose**: Governs the perioperative operational milestones of a surgical procedure within the operating theatre suite.
- **State Progression Lifecycle**: Case Called → In Anaesthetic Bay → Anaesthesia Commenced → Patient in Theatre → Knife-to-Skin → Procedure Finished → In PACU Recovery → Ward Handover Complete
- **Key State Dispositions**:
  - `CaseCalled`: Theatre team requests ward to send patient for surgery.
  - `InAnaestheticBay`: Patient arrives in theatre holding and identity/consent verified.
  - `AnaesthesiaCommenced`: Anaesthetic induction underway.
  - `PatientinTheatre`: Patient transferred onto operating table.
  - `Knife-to-Skin`: Surgical incision initiated (operative start).
  - `ProcedureFinished`: Wound closure completed and dressings applied.
  - `InPACURecovery`: Patient monitored in post-anaesthesia care unit.
  - `WardHandoverComplete`: Post-operative clinical handover to inpatient ward completed.

4.8 Clinic / Operational Queue Progression Process

- **Owning Capability**: `Clinic&PracticeOperations`
- **Process Purpose**: Governs the movement and state tracking of outpatients through an ambulatory specialty clinic session.
- **State Progression Lifecycle**: Scheduled → Patient Arrived → Checked-In → Roomed / Pre-Consult → Consultation In-Progress → Consultation Concluded → Departed
- **Key State Dispositions**:
  - `Scheduled`: Appointment booked on session list.
  - `PatientArrived`: Patient enters waiting area.
  - `Checked-In`: Reception verifies demographics and marks patient ready.

- `Roomed/Pre-Consult`: Nursing staff record baseline vitals and room patient.
- `ConsultationIn-Progress`: Clinician actively conducting consultation.
- `ConsultationConcluded`: Clinical discussion finished; follow-up orders placed.
- `Departed`: Patient checks out and departs clinic facility.

4.9 Bed Turnover Process

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Process Purpose**: Governs the rapid physical turnover, cleaning, sanitisation, and re-allocation of inpatient and emergency care-places.
- **State Progression Lifecycle**: Bed Vacated → Cleaning Dispatched → Sanitisation In-Progress → Terminal Cleaning Verified → Bed Ready / Available → Bed Allocated / Occupied
- **Key State Dispositions**:
  - `BedVacated`: Prior patient discharged or transferred.
  - `CleaningDispatched`: Cleaning job allocated to environmental services team.
  - `SanitisationIn-Progress`: Ward cleaning or isolation decontamination underway.
  - `TerminalCleaningVerified`: Ward coordinator or supervisor verifies care-place readiness.
  - `BedReady/Available`: Bed marked available for incoming patient allocation.
  - `BedAllocated/Occupied`: Incoming patient allocated and physically placed in bed.

4.10 Operational Work Progression Process

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Process Purpose**: Governs the dispatch, assignment, execution, and completion of facility operational jobs (e.g., equipment moves, waste removal, linen supply).
- **State Progression Lifecycle**: Work Requested → Queued in Dispatch → Assigned to Worker → Job Accepted → Work In-Progress → Work Completed / Aborted
- **Key State Dispositions**:
  - `WorkRequested`: Facility support task submitted by clinical unit.
  - `QueuedinDispatch`: Job prioritized in operational queue.
  - `AssignedtoWorker`: Job allocated to specific staff member or team.
  - `JobAccepted`: Worker acknowledges dispatch on mobile device.
  - `WorkIn-Progress`: Task actively being executed.
  - `WorkCompleted/Aborted`: Job marked finished or formally cancelled with reason.

4.11 Patient Transport Process

- **Owning Capability**: `PatientTransport`
- **Process Purpose**: Governs the physical portering and transport of patients between hospital departments, wards, diagnostic suites, and external facilities.
- **State Progression Lifecycle**: Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Clinical Handover Complete
- **Key State Dispositions**:
  - `TransportRequested`: Clinical unit books wheelchair, bed, or ambulance transport.
  - `PorterDispatched`: Wardsperson assigned and en route to pickup location.
  - `PatientCollected`: Patient identity verified and transfer initiated.
  - `In-Transit`: Patient actively moving through facility corridors or ambulance transit.
  - `DeliveredatDestination`: Patient arrives at target diagnostic suite or ward.
  - `ClinicalHandoverComplete`: Safe custody and clinical briefing transferred to receiving staff.

4.12 Specimen Transport Process

- **Owning Capability**: `ClinicalLogisticsCoordination`

- **Process Purpose**: Governs the physical courier chain-of-custody, transport, and delivery of pathology bio-specimens, blood products, and surgical biopsies.
- **State Progression Lifecycle**: Specimen Packaged → Courier Dispatched → Chain-of-Custody Signed → In-Transit (Monitored) → Lab Ingress Received → Specimen Accepted
- **Key State Dispositions**:
  - `SpecimenPackaged`: Clinical unit packages bio-sample with barcode manifest.
  - `CourierDispatched`: Courier route assigned for pickup.
  - `Chain-of-CustodySigned`: Courier scans barcode and accepts legal custody.
  - `In-Transit(Monitored)`: Specimen in transit under temperature/time constraints.
  - `LabIngressReceived`: Central pathology specimen reception logs physical receipt.
  - `SpecimenAccepted`: Specimen integrity verified and passed to analytical track.

4.13 Discharge Progression Process

- **Owning Capability**: `DischargeManagement`
- **Process Purpose**: Governs the multidisciplinary coordination of patient discharge planning, pharmacy reconciliation, transport, and community handover.
- **State Progression Lifecycle**: Discharge Planning Initiated → Clinical Readiness Confirmed → Medications Reconciled → Transport & Services Booked → Discharge Summary Signed → Physically Departed (Illustrative preparation; publication, authorisation and exit distinct)
- **Key State Dispositions**:
  - `DischargePlanningInitiated`: Estimated Date of Discharge (EDD) set on admission.
  - `ClinicalReadinessConfirmed`: Attending medical team declares patient fit for discharge.
  - `MedicationsReconciled`: Hospital pharmacy reconciles and dispenses discharge medications.
  - `Transport&ServicesBooked`: Community nursing, home equipment, and patient transport booked.
  - `DischargeSummarySigned`: Applicable signed summary is available/published and may be communicated before departure where the Business behaviour requires it; signing, finalisation, publication and discharge authorisation remain distinct.
  - `PhysicallyDeparted`: Confirmed physical exit triggers departure notifications, updated bed-state communication and finalised-summary dispatch to external care providers under `FEAT-HSO-29`; care-place turnover follows its own responsibility.
- **Timing reconciliation**: Preparation, information availability/publication, applicable discharge authorisation and physical exit are distinct. Earlier preparation/communication is permitted; it does not realise a Feature triggered by confirmed exit. This illustrated local pathway establishes neither universal publication timing nor a universal discharge state machine, and adds no inferred publication event or authorisation state.
- **Preserved uncertainty**: Summary signing and finalisation are not universal equivalents. Detailed applicability, discharge-authority workflows and timing beyond the established preparation/confirmed-exit distinction remain unestablished.

5. Horizontal Workflow Coordination Processes

5.14 Work Order Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Doing** — physical or operational task executed by healthcare staff.
- **State Progression Lifecycle**: Created → Ready → Dispatched → Accepted → In-Progress → Completed / Failed / Cancelled
- **Key State Dispositions**:
  - `Created`: Work order instantiated with scope, target, and required skills.
  - `Ready`: Preconditions satisfied; available for assignment.
  - `Dispatched`: Sent to target individual or worker group queue.
  - `Accepted`: Worker claims task.

- `In-Progress`: Execution underway.
- `Completed`: Task finished with recorded completion telemetry.

5.15 To Do Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Reviewing / Updating / Deciding** — clinical review, document countersignature, or administrative authorization task.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    I["Issued"] --> B["In-Inbox"]
    B --> R["Opened / Under-Review"]
    R --> A["Actioned / Decided"]
    D["Dismissed / Delegated<br/>Disposition/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>Relationship of dismissal/delegation
    ↪  to action/decision is unestablished."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,D note;
```

- **Key State Dispositions**:
  - `Issued`: To Do generated (e.g., review abnormal potassium result).
  - `In-Inbox`: Displayed in practitioner's actionable task list.
  - `Opened/Under-Review`: Clinician inspecting relevant clinical context.
  - `Actioned/Decided`: Decision executed (e.g., signed off, order placed, acknowledged).
  - `Dismissed/Delegated`: Task reassigned to registrar or dismissed with comment.
- **Preserved uncertainty**: Whether dismissal/delegation is an alternative disposition, a subsequent action or a scoped variant remains unresolved; its disconnected diagram node establishes no transition or position in the illustrative progression.

5.16 Synthetic Task Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Non-Human Executable Work** — automated system workflows, batch syndications, or policy evaluation tasks.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    D["Draft / Scheduled"] --> E["Executing"]
    E --> W["Awaiting-Dependency"]
    W --> C["Completed"]
    F["Progression Stalled / Failed<br/>Outcome/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>No Completed-to-Stalled/Failed
    ↪  transition is established."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,F note;
```

- **Key State Dispositions**:
  - `Draft/Scheduled`: Automated task instantiated and scheduled for processing.
  - `Executing`: System actively processing task.
  - `Awaiting-Dependency`: Paused awaiting asynchronous external reply or event.
  - `Completed`: Automated execution succeeded and output artifact produced.
  - `ProgressionStalled/Failed`: Transient fault triggers scheduled recovery attempt; permanent failure records failure evidence.
- **Preserved uncertainty**: Whether stalled/failed is an alternative outcome, reopening/post-completion behaviour or a presentation defect remains unresolved. Its disconnected diagram node does not make success and failure equivalent or establish post-completion transitions.

6. Health Service Assurance Processes

The approved human review decisions on 2026-10-08 establish exactly the following three assurance Processes, building on the assurance Strategy and the two established assurance Roles and five Functions. **Element Type: Business Process (PR)** applies to each. Capability ownership below follows the established responsibility; **Feature association is not established** and **Canonical IDs remain unresolved** because Capability Tier, complete ancestry and local identifier allocation remain unestablished. Catalogue numbering is navigation, not identifier allocation.

6.1 Assurance Process Design

- **Canonical Name**: `AssuranceProcessDesign`.
- **Owning Capability**: Assurance Design.
- **Process Purpose**: Establishes and maintains how satisfaction of a governed requirement, constraint or expected behaviour is to be assured.
- **Principal Performing Role**: Service Assurance Modeller.
- **Participating Functions**: Design Service Assurance and Manage Assurance Criteria. Manage Assurance Criteria retains ownership within Assurance Criteria Management; its participation does not subsume that Capability's criteria lifecycle or transfer its information responsibility to Assurance Design.
- **Conceptual Output**: Assurance Definition, retaining its conceptual Business Architecture output/boundary treatment.
- **Boundary**: Governance Authority establishes what is authoritative. Design does not grant Service Assurance Modeller approval authority. Detailed Assurance Definition lifecycle, approval/publication workflows, criteria lifecycle and information modelling remain unresolved. Participation of the two Functions does not establish a cross-capability lifecycle or a new exchange/Service between their owners.

6.2 Assurance Process Execution

- **Canonical Name**: `AssuranceProcessExecution`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Performs defined assurance for an identifiable governed subject in accordance with an applicable Assurance Definition and establishes an Assurance Finding / Conclusion from applicable trustworthy evidence.
- **Principal Performing Role**: Service Guardian.
- **Participating Functions**: Establish Assurance Context, Assess Assurance Evidence and Adjudicate Assurance Assessment, all retaining Governed Assurance ownership.
- **Conceptual Basis and Output**: Applicable Assurance Definition plus identifiable governed subject; Assurance Finding / Conclusion established through adjudication where sufficient trustworthy evidence permits. Correct execution with insufficient evidence does not imply satisfaction or non-satisfaction or force a conclusion.
- **Boundary**: Function completion does not define Process lifecycle states. No Context Established, Evidence Assessed, Adjudicated or Concluded states are inferred. Detailed lifecycle states, transitions and initiation mechanics remain unresolved. Requesting this Process does not grant control of evidence assessment, adjudication or the resulting finding/conclusion.

RAD diagram: textual source notation.

```
graph TD
    DEF["Applicable Assurance Definition<br/>Conceptual basis"] -->|Assurance basis| EXEC["PR: Assurance Process
    ↪  Execution<br/>Owner: Governed Assurance"]
    SUBJECT["Identifiable Governed Subject"] -->|Subject of assurance| EXEC
    EXEC -->|Participating Function| CONTEXT["FN: Establish Assurance Context"]
    EXEC -->|Participating Function| ASSESS["FN: Assess Assurance Evidence"]
    EXEC -->|Participating Function| ADJ["FN: Adjudicate Assurance Assessment"]
    EXEC -->|Establishes through adjudication| FINDING["Assurance Finding / Conclusion"]
```

6.3 Assurance Process Reporting / Communication

- **Canonical Name**: `AssuranceProcessReporting/Communication`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Communicates applicable established Assurance Findings and Conclusions to legitimate recipients according to their responsibilities, authority and information requirements, without altering the established assurance outcome or assuming responsibility for the recipient's subsequent action.
- **Starting Basis**: An already established Assurance Finding / Conclusion. This Process does not perform assessment or adjudication, alter or reinterpret the established outcome.
- **Communication Participation**: Service Guardian participates through Service Assurance Outcome Communication, associated with Communicate Service Assurance Outcome. No additional Function is introduced to populate this Process.
- **Boundary**: Reporting / Communication is not limited to documentary report production; an appropriate Business Interaction may communicate the outcome without a report document. It assumes no operational response, remediation, escalation, policy/compliance response, process improvement, clinical management, clinical judgement or other recipient action. Detailed communication progression and completion semantics remain unresolved.

6.4 Process-Family Relationship and Preserved Boundaries

RAD diagram: textual source notation.

```
graph TD
    DESIGN["PR: Assurance Process Design<br/>Owner: Assurance Design"] -->|Conceptual output| DEF["Assurance
    ↪  Definition"]
    DEF -->|Reusable assurance basis| EXEC["PR: Assurance Process Execution<br/>Owner: Governed Assurance"]
    EXEC -->|Established through adjudication| FINDING["Assurance Finding / Conclusion"]
    FINDING -->|Already established outcome| COMM["PR: Assurance Process Reporting / Communication<br/>Owner:
    ↪  Governed Assurance"]
```

Both diagrams express semantic Business Architecture relationships, not lifecycle states, mandatory runtime topology, application orchestration, deployment sequence, component structure, message flow or a complete implementation workflow. An applicable Assurance Definition may be reused by multiple Execution instances; each assurance activity does not require a newly executed Design instance. Using the modelling output establishes a responsibility/information dependency, not a Modeller-to-Guardian Interaction or Collaboration.

> **Governed Assurance does not recursively assure its own execution. Assurance activity execution integrity is provided by the established activity execution framework.**

Managed progression of assurance activity does not make that execution another Service Assurance subject. No recursive assurance Process, Function, Service or conclusion is established; the activity execution framework reference allocates no solution component. System Steward's status observation is management/stewardship of progression, not assurance of assurance. See the non-recursion boundary.

The sixteen pre-existing Processes retain their owners, definitions and state models. Clinical review, sign-off, countersignature and clinical decisions remain with applicable clinical processes/authorities; To Do completion is not an independent assurance conclusion. Generic Work Order, To Do and Synthetic Task progression do not supply assurance lifecycle states or an assurance execution allocation.

Service Guardian may manage its own assurance progression without managing its subject. Neither assurance Role acquires the subject's progression, assignment, delegation, failure/recovery, operational escalation, remediation or improvement responsibility. Execution failure/indeterminate outcome remain distinct from correctly executed assurance with insufficient evidence. Remaining decisions include lifecycle states/transitions, initiation mechanics, detailed information semantics and response mechanisms; the three Process names and purposes are resolved.

44. Health Service Assurance — Bounded Business Architecture Derivation

RAD source: `docs/markdown/03-business-architecture/behaviours/health-service-assurance.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Health Service Assurance — Bounded Business Architecture Derivation

1. Authority and Derivation Scope

This view carries the approved Domain02 Health Service Assurance derivation into Domain03. Its upstream responsibility chain is REQ-FND-005 — Independent Assurance of Governed Activity → BC-18 Health Service Assurance → **Assurance Design**, **Assurance Criteria Management** and **Governed Assurance**, with the approved collaborative Enterprise Capability contributions including **EC-14 Service Guardian**.

AX-14 requires the responsibility distinctions below to remain explicit. AX-17 and the Domain03 metamodel require established responsibility to be distinguished from unestablished naming, decomposition, exposure, progression and authority relationships. AX-06, AX-07, AX-08 and AX-09 continue to govern information authority, governed access, meaningful evidence and contextual evidence selection. AX-15 governs operational uncertainty without making it equivalent to insufficient assurance evidence.

The approved human review decisions on 2026-10-08 extend the previous bounded derivation with **Service Assurance Modeller** and **Service Guardian** Business Roles and exactly five Business Functions: **Design Service Assurance**, **Manage Assurance Criteria**, **Establish Assurance Context**, **Assess Assurance Evidence** and **Adjudicate Assurance Assessment**. Their capability ownership and Role performance are documented in §3. The assurance Role previously named Guardian is now Service Guardian; legal/personal-welfare Guardian remains unchanged. These naming and Function-decomposition decisions are resolved; further decomposition is not approved.

The next approved human review decisions on 2026-10-08 establish exactly three Business Processes, three Business Services and three Business Interactions, with the approved consumer/participant relationships. They also establish the Financial Governance deliberate exclusion and assurance non-recursion boundary. The five approved Functions are preserved without reinterpretation or further decomposition.

The three assurance capabilities remain established responsibility contexts with unresolved contextual-view placement, Capability Tier, complete ancestry, Feature decomposition and structural Canonical IDs. This document is a capability-scoped view, not a sixth contextual view or an identifier ancestor. Specific Actors and Collaborations remain unallocated; detailed Process states/transitions, information models and implementation remain unresolved.

EC-14 Service Guardian is an Enterprise Capability. Service Guardian is also the intentional canonical name of a distinct Business Role. Shared terminology does not equate their architectural types or make either a software component. The approved human review decisions supply the Business Architecture relationships; they are not inferred from the EC contribution model or its explanatory sequence. Previous completion reports supply execution context only and are not architectural authority.

2. Governance, Management, Guardianship and Clinical Authority

RAD source listing (presentation only; no inferred ancestry).

Governance defines

↓

Management performs

↓

Guardianship assures

This expresses semantic responsibilities, not organisational structure, a compulsory execution sequence or technical topology.

Responsibility	Business meaning	Preserved boundary
Governance	Defines applicable requirements, obligations, authority, constraints, controls, reporting requirements and checkpoints.	Establishes the normative envelope without taking over the operational means or progression of the governed activity. Subject-specific governance remains intrinsic to its established subject capability.
Operational management	Performs or manages the subject activity within its constraints, including monitoring for progression, assignment, delegation, operational escalation, response, remediation and expected failure/recovery.	Monitoring for operational completion or deciding what happens next does not establish independent assurance.
Service Assurance modelling	Service Assurance Modeller determines and models how satisfaction of governed requirements, constraints and expected behaviours is to be assured within Assurance Design and Assurance Criteria Management.	Modelling does not inherently approve the governing requirement, Assurance Definition or Assurance Criteria, perform independent assurance, or manage the subject.
Guardianship / governed assurance	Service Guardian independently evaluates governed activity, information, state or outcomes against applicable criteria and establishes findings and conclusions from sufficient trustworthy evidence.	Service Guardian may manage its own assurance activity; it does not thereby perform or manage the subject, direct its operational response or confer clinical authority.
Clinical review / clinical assurance	Exercises the applicable clinical process and authority's responsibility for clinical evaluation and professional judgement.	Neither assurance Role establishes clinical adequacy, clinical correctness or professional clinical judgement. Clinical quality, safety and improvement retain their existing responsibilities.

Governance Authority establishes what is authoritative. Service Assurance Modeller models how satisfaction will be assured. Service Guardian independently establishes what the applicable evidence demonstrates. Governance Authority references the existing mandated authority Roles, including Policy Authority for policy and Regulator for statutory authority, with existing steward mandates preserved. No new Governance Authority Role or extension of those mandates is established. Modelling does not grant approval authority for the governing requirement, Assurance Definition or Assurance Criteria; exact approval allocation/workflows remain unresolved.

RAD diagram: textual source notation.

```
graph TD
    GOV["Governance Authority<br/>Existing mandated authority Roles"] -->|Establishes authoritative basis|
    ↪  MOD["Business Role<br/>Service Assurance Modeller"]
```

```
MOD -->|Performs| DESIGN["Function: Design Service Assurance<br/>Owner: Assurance Design"]
MOD -->|Performs| CRITERIA["Function: Manage Assurance Criteria<br/>Owner: Assurance Criteria Management"]
MOD -->|Models| DEF["Assurance Definition<br/>Conceptual modelling output"]
DEF -->|Modelling boundary for assurance| GUARD["Business Role<br/>Service Guardian"]
GUARD -->|Performs| CONTEXT["Function: Establish Assurance Context<br/>Owner: Governed Assurance"]
GUARD -->|Performs| ASSESS["Function: Assess Assurance Evidence<br/>Owner: Governed Assurance"]
GUARD -->|Performs| ADJ["Function: Adjudicate Assurance Assessment<br/>Owner: Governed Assurance"]
ADJ -->|Establishes| FINDING["Assurance Finding / Conclusion"]
```

This is a responsibility/semantic model, not mandatory execution topology, organisational or application structure, deployment topology or a complete Business Process. The output labels do not introduce Information Objects or a formal information model. The diagram distinguishes performance through a Role from Function ownership by a Capability and does not define process ordering or an assurance initiation/approval exchange.

A real-world Actor may fulfil different Roles in different contexts where governance permits. That possibility does not relax REQ-FND-005: assurance progression and conclusion must remain independently governed rather than controlled by the performer or manager of the subject. The subject may contribute evidence but must not solely determine, suppress, manufacture or retrospectively alter its assurance conclusion. Evidence dependency and control dependency remain distinct.

Neither Service Assurance Modeller nor Service Guardian inherently possesses clinical authority or replaces professional clinical judgement, clinical peer review or clinical governance. Information and behaviour concerning clinical activity may be subject to governed assurance where an explicit applicable requirement establishes that concern. This does not constitute **Clinical Services Delivery Assurance** or transfer clinical-care responsibility to Guardianship or Harmonia. The approved clinical boundary, Service Delivery care-enablement principle, and existing clinical review responsibilities remain controlling. No generic clinical-assurance authority Role is derived.

Neither assurance Role manages the subject merely because it designs or performs assurance. Service Guardian may manage progression of its own assurance activity without assuming progression of its subject. Findings may cause another responsible party to initiate management, escalation, remediation or other operational action; no such response responsibility is allocated to Service Guardian.

3. Direct Capability-Scoped Responsibility Consequences

The following five Functions are approved Business Architecture. Each is behaviour within its established owning Capability, performed through the stated Business Role. **Element Type: Business Function (FN)** applies to each. **Canonical IDs remain unresolved** because Capability Tier, complete ancestry and local identifier allocation are unestablished. **Feature association is not established** for any of these Functions; no Feature or structural ID is invented to anchor them. Function ownership does not transfer source-information ownership to an assurance Role or Capability.

3.1 Assurance Design — Service Assurance Modeller

- **Owning Capability**: Assurance Design.
- **Performing Business Role**: Service Assurance Modeller.

Design Service Assurance

- **Canonical Name**: `DesignServiceAssurance`.
- **Definition**: Defines how satisfaction of a governed requirement, constraint or expected behaviour is to be independently established, including the applicable assurance disposition, assurance criteria, required evidence and evaluation expectations.

- **Boundary**: Concerns assurance design. It does not establish the underlying governance requirement, grant authority to it, perform the resulting assurance, manage the subject activity or prescribe application or implementation mechanisms. Modelling the resulting Assurance Definition does not grant approval authority. The Strategy peer relationship to behaviour design and failure/recovery design creates no additional Capability or Function.

3.2 Assurance Criteria Management — Service Assurance Modeller

- **Owning Capability**: Assurance Criteria Management.
- **Performing Business Role**: Service Assurance Modeller.

Manage Assurance Criteria

- **Canonical Name**: `ManageAssuranceCriteria`.
- **Definition**: Establishes and manages reusable, governed and temporally identifiable assurance criteria through which governed subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These examples are not an exhaustive taxonomy. Criteria may have lifecycle, version and temporal applicability.
- **Boundary**: Managing a criterion does not transfer authority for the governing requirement from which it may derive. Criteria may operationalise a requirement without silently altering, weakening, strengthening or replacing it; not every criterion must derive from formal Governance. Modelling/managing criteria does not inherently confer approval authority. Detailed criterion lifecycle states and approval workflows remain unresolved.

3.3 Governed Assurance — Service Guardian

- **Owning Capability**: Governed Assurance.
- **Performing Business Role**: Service Guardian.

Establish Assurance Context

- **Canonical Name**: `EstablishAssuranceContext`.
- **Definition**: Establishes the governed subject, applicable assurance purpose and criteria, relevant temporal context, and contextual association of sufficient trustworthy source information as evidence required for an assurance activity.
- **Scope**: Includes the evidence-assembly responsibility previously described as Assurance Evidence Assembly; no separate evidence-assembly Function is established. Sufficiency is required for conclusion, not assumed merely because context is established; unavailable or insufficient evidence remains explicit.
- **Information Boundary**: Source information remains owned by its responsible source Capability. Its use as evidence does not transfer ownership or originating authority. Governed Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Evidence association and applicable criteria may each be temporal and version-specific; association distinguishes the information state relevant to assurance from its current state.
- **Realisation Boundary**: Uses generic EC-02 context-management capability together with EC-14 assurance-specific semantics as documented in §4.1. Establishment of Assurance Context prescribes no copying, persistence, aggregation, caching or other technical mechanism.

Assess Assurance Evidence

- **Canonical Name**: `AssessAssuranceEvidence`.
- **Definition**: Evaluates the available trustworthy evidence against the applicable assurance criteria and establishes what that evidence demonstrates for each relevant criterion or assessment concern.
- **Boundary**: Assessment remains distinct from adjudication. It may identify satisfaction, exception, insufficiency or other criterion-level results where applicable; no universal result taxonomy is established. Quantitative, qualitative, compound, temporal and confidence-bearing assessment remain possible. **Insufficient evidence is not equivalent to either satisfaction or non-satisfaction.**

Adjudicate Assurance Assessment

- **Canonical Name**: `AdjudicateAssuranceAssessment`.
- **Definition**: Determines the assurance finding or conclusion that follows from the assessed evidence and applicable assurance criteria.
- **Boundary**: Establishment of the finding/conclusion is part of adjudication. It remains distinct from evidence assessment, operational response, remediation, escalation, workflow progression and clinical judgement. No separate conclusion-establishment or reporting Function is introduced merely because a conclusion is established or may be communicated.

3.4 Preserved Assurance Obligations

The approved assurance dispositions remain explicit governed decisions. **Not Assurable** does not remove the requirement; **Not Worth Assuring** does not make it optional. Disposition does not require specific assurance of every individual activity, information item or outcome.

The approved generic processing assurance obligation constrains Harmonia-managed workflow behaviour. Evaluating business progression, completion, timeliness, pathways, failure/recovery and outcomes for assurance is distinct from monitoring to progress that work. The existing operational Processes and Functions retain their owners; neither successful completion nor an operationally healthy environment establishes an assurance conclusion. No final criterion catalogue or applicability model is supplied by these examples.

Assessment asks what the evidence demonstrates against applicable criteria; adjudication establishes what finding or conclusion follows. The approved decisions now establish these as distinct Functions performed through Service Guardian; they do not require separate Roles or process stages. Assurance is not universally reducible to PASS / FAIL / UNKNOWN. An assurance activity may execute correctly while lacking enough evidence to conclude. That insufficiency is distinct from its own execution failure or indeterminate execution outcome and must not imply satisfaction or non-satisfaction.

4. Information and Dependency Consequences

The Business Information Responsibility material records the directly supported demarcations: Assurance Design defines how satisfaction will be assured; Assurance Criteria Management manages criteria; Governed Assurance owns contextual evidentiary association and its findings/conclusions. Underlying source information remains owned and managed by its established capability. Evaluating it does not acquire its originating authority.

Evidence association must distinguish the relevant information state from its current state where temporal/version-specific inclusion matters. Evaluation must use temporally appropriate evidence and criteria. Evidence collection, provenance, audit information and operational completion are not themselves assurance conclusions. No assurance Information Object, outcome taxonomy, representation, custody, retention rule or persistence structure is defined here.

An evidence need does not identify a Business Service/provider or authorise a new consumption edge. The dependency assessment preserves that distinction, including the prohibition

on subject control of assurance progression or conclusion. The approved Enterprise Capability contribution matrix remains a Strategy contribution model, not a Business Service consumption matrix.

4.1 Context Management Contribution

EC-02 Context Management supplies generic establishment, validation, propagation and preservation of meaningful context. EC-14 Service Guardian supplies assurance-specific subject/purpose, criterion applicability and contextual evidentiary-association semantics. Their contributions together support **Establish Assurance Context**, owned by Governed Assurance and performed through the Service Guardian Business Role.

RAD diagram: textual source notation.

```
graph TD
    CONTEXT["Enterprise Capability<br/>EC-02 Context Management"] -->|Generic context semantics| FN["Business
    ↪  Function<br/>Establish Assurance Context<br/>Owner: Governed Assurance"]
    ASSURANCE["Enterprise Capability<br/>EC-14 Service Guardian"] -->|Assurance-specific semantics| FN
```

This documents the approved semantic contribution relationship. In the Domain02 collaborative contribution matrix, EC-02 and EC-14 each contribute directly to Governed Assurance. Neither is a sole realiser or Function owner; the other approved capability contributions remain intact. This view is not an exhaustive Function-to-EC allocation matrix, a new Business Service consumption edge or an application/runtime allocation. How context establishment is realised belongs to later architecture; no loader/unloader mechanism or implementation pattern is derived.

4.2 Assurance Definition — Conceptual Modelling Output

Assurance Definition describes the conceptual output/boundary between Service Assurance modelling and governed assurance: how satisfaction is to be assured, including assurance disposition, applicable criteria, required evidence and evaluation expectations. This output description follows directly from **Design Service Assurance** within **Assurance Design**, performed through **Service Assurance Modeller**; governed assurance uses that modelling boundary without transferring governance authority or establishing an exposed Service or approval exchange.

The existing Domain03 rules permit documenting this business output and its responsibility context. They do not establish its formal information structure, identity, lifecycle, representation or approval model. The term therefore remains a conceptual responsibility/output description, not a newly allocated formal Business Information element, Information Object or Data Object. Formal information modelling is deferred. Modelling an Assurance Definition does not make it authoritative or grant its modeller approval authority; applicable governance authority must establish that standing.

5. Approved Governed Assurance Business Services

Exactly three assurance Services are established below. **Element Type: Business Service (SV)** applies to each; each is owned/exposed by **Governed Assurance** with Service Guardian participating through its associated Business Interaction. Identifiable consumers/recipients outside that responsibility boundary justify the exposure under the metamodel. **Feature association is not established** and **Canonical IDs remain unresolved**; no Capability Tier, ancestry or local code is invented.

The Services expose approved request, activity-progression observation and established-outcome communication behaviour. They do not mechanically correspond one-to-one with the five Functions, nor introduce new request, status or reporting Functions. Detailed internal Service-to-Function mappings and contracts beyond the approved behaviour remain unestablished. Status observation concerns assurance's own permitted activity progression, not assessment of the subject or a new status Process.

5.1 Request Service Assurance

- **Canonical Name**: `RequestServiceAssurance`.
- **Definition**: Enables an authorised consumer to request that Service Assurance be performed for an applicable governed subject in accordance with an established Assurance Definition.
- **Approved Consumers**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Request, with Service Guardian and one authorised requesting Role.
- **Process Relationship**: Requests Assurance Process Execution under an applicable established Assurance Definition. It does not require a new Assurance Process Design instance for each request.
- **Independence Boundary**: The requesting Role identifies the assurance need within its responsibility and authority. Requesting does not grant authority to define or alter the Assurance Definition, alter Assurance Criteria, direct evidence assessment, determine adjudication or control the resulting Assurance Finding / Conclusion. REQ-FND-005 control independence remains applicable, including where a requester performs or manages the subject. Detailed payloads, contracts, initiation mechanics and states remain unresolved.

5.2 Request Service Assurance Status

- **Canonical Name**: `RequestServiceAssuranceStatus`.
- **Definition**: Enables the System Steward to obtain the current business progression state of an identifiable Service Assurance activity for stewardship purposes.
- **Only Approved Consumer**: `SystemSteward`.
- **Associated Interaction**: Service Assurance Status, between System Steward and Service Guardian.
- **Status Boundary**: Concerns **activity progression state**. It does not communicate provisional findings, incomplete assessment results as conclusions, predicted outcomes or unadjudicated conclusions. Other requesting Roles do not gain status access merely because they requested assurance. No detailed activity-status taxonomy is established.

Service Assurance Status answers "where is the assurance activity in its progression?" It does not answer "what has the assurance established?" The latter requires an adjudicated Assurance Finding / Conclusion and subsequent outcome communication; neither status nor execution completion establishes that conclusion.

Management Monitoring evaluates activity state in order to progress or manage activity. Assurance evaluates activity against governing criteria in order to establish an assurance conclusion. System Steward's use of this Service is stewardship / management observation of assurance progression, not itself assurance or recursive assurance. The non-recursion boundary in §7.2 applies.

5.3 Communicate Service Assurance Outcome

- **Canonical Name**: `CommunicateServiceAssuranceOutcome`.
- **Definition**: Communicates applicable established Assurance Findings and Conclusions to authorised recipients according to their responsibilities, authority and information requirements.
- **Approved Potential Recipients**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Outcome Communication, with Service Guardian and one or more authorised recipient Roles.
- **Process Relationship**: Exposes the communication behaviour of Assurance Process Reporting / Communication, beginning from an already established finding/conclusion. It does not perform assessment or adjudication or alter/reinterpret the established outcome. A documentary report is not required.

- **Recipient Boundary**: No mandatory broadcast is established. A particular outcome may be communicated to one, some or all legitimate recipients according to applicability, authority, responsibility and information requirements. Requester, status consumer and outcome recipient need not be the same Role. Communication does not transfer responsibility for recipients' interpretation within their authority, operational/clinical management, remediation, escalation, policy/compliance response, process improvement or other subsequent action to Service Guardian.

5.4 Approved Consumer and Recipient Concerns

The existing Role definitions and mandates remain controlling. These are approved consumer/recipient concerns, not exhaustive Role responsibilities or new Roles. Each use requires applicable authority; Role eligibility alone does not grant unrestricted information access.

Business Role	Request Service Assurance	Request Service Assurance Status	Potential recipient of Communicate Service Assurance Outcome	Principal assurance concern
Care Coordinator	Approved within authority	Not approved	Approved where applicable/ authorised	Clinical management
Service Coordinator	Approved within authority	Not approved	Approved where applicable/ authorised	Operational management and potentially task / people process improvement
Regulator	Approved within authority	Not approved	Approved where applicable/ authorised	Service governance / compliance
Policy Authority	Approved within authority	Not approved	Approved where applicable/ authorised	Service governance / compliance and policy effectiveness / improvement
System Steward	Approved within authority	Only approved consumer	Approved where applicable/ authorised	ICT process improvement and stewardship of assurance activity progression

Care Coordinator's request or receipt may inform Clinical Management; clinical interpretation, judgement and action remain with the applicable clinical authority. Service Coordinator retains operational management/improvement responsibilities; System Steward's status access does not make Service Guardian responsible for task progression, assignment, delegation, operational escalation, remediation or process improvement. Regulator and Policy Authority retain their established governance/compliance mandates. None of these consumer relationships transfers source-information ownership, governance authority or control of assurance assessment/adjudication.

6. Additional Business Elements Not Yet Established

The two assurance Roles, five Functions, three Processes, three Services, three Interactions and approved Role relationships are established. The following further concerns remain explicitly **unresolved and non-authoritative**; their inclusion does not add them to a catalogue. Financial Governance's scope exclusion and assurance non-recursion are settled boundaries, not gaps.

Candidate concern assessed	What approved architecture establishes	Architectural decision required before adding an element
Further Function decomposition / Feature association	Exactly the five approved Functions in §3, including evidence assembly within context establishment and conclusion establishment within adjudication.	Additional Functions and Feature associations require a further decision. No separate applicability/criteria-resolution, evidence-assembly, conclusion-establishment, reporting, remediation or escalation Function is approved.
Further Business Services / contracts	Exactly the three Services in §5 with their approved consumers/ recipients, owner and behaviour.	Additional criteria/evidence Services, exact evidence/criteria providers, detailed Service contracts and internal Service-to-Function mappings are unestablished. No extra Service is inferred from reportability, reuse or evidence need.
Detailed Process progression	Assurance Process Design, Execution and Reporting / Communication have established purposes, participating Functions where approved, conceptual boundaries and ownership.	Detailed lifecycle states/transitions, initiation mechanics, completion/ disposition semantics, criteria lifecycle and approval/publication workflows remain unresolved. Function completion, EC-14's sequence and existing coordination lifecycles do not supply assurance states.
Further Business Interactions	Service Assurance Request, Status and Outcome Communication with approved Role participants and associated Services.	Evidence acquisition/exchange Interactions remain unresolved; source-owned information may be available through existing governed information access. No Evidence Contribution or Modeller-to-Guardian Interaction is created from evidence association or use of an Assurance Definition. Further exchange commitments require separate approval.
Assurance Role collaborations	Roles participate in the three approved Interactions; capability realisation is collaborative.	Establish an enduring, structured Actor/ Role association with a defined purpose and permitted control relationships. Interaction does not mechanically imply Collaboration; no existing membership or new assurance collective is established.
Further responsibility relationships / dependencies	Approved request/status/ outcome Role relationships, governance mandates and independence; source ownership, operational management and clinical authority remain separate.	Detailed Actor eligibility/mandates beyond approved Roles, coverage/applicability, initiation mechanics, exact approval allocation, evidence/criteria providers, findings-to-operational-response mechanisms and exact authorised external financial-governance recipients remain unresolved. Financial governance responsibility itself is deliberately excluded.
Further Business Information responsibilities	Responsibility demarcations and contextual/temporal evidence obligations in §4; conceptual Assurance Definition; progression status distinct from established findings/conclusions and their communication.	Detailed request/status/outcome information semantics, formal Assurance Definition modelling, further information concepts/asset identity, custody/lifecycle, criterion catalogues, outcome taxonomies, confidence models and retention remain unresolved. No formal information element or source-information reassignment is inferred.

6.1 Previous Questions Resolved by Approved Human Review

Previous question	Approved resolution
Legal Guardian / assurance Guardian naming collision	**Resolved:** legal/personal-welfare Guardian remains intact; governed assurance uses the unambiguous **Service Guardian** Business Role. EC-14 retains the intentional shared name as a distinct Enterprise Capability.
Who models how satisfaction will be assured?	**Resolved: Service Assurance Modeller** performs Design Service Assurance within Assurance Design and Manage Assurance Criteria within Assurance Criteria Management, without inherent governance/approval authority.
Function names and bounded decomposition	**Resolved for this stage:** exactly five Functions are established in §3. Evidence assembly is included in Establish Assurance Context; finding/conclusion establishment is included in Adjudicate Assurance Assessment. No separate reporting Function is established.
Generic context management / assurance-specific context relationship	**Resolved to the approved extent:** EC-02 and EC-14 contribute together to Establish Assurance Context; sole realisation, Service exposure and implementation allocation are not implied.
Assurance Process names and purposes	**Resolved:** exactly Assurance Process Design, Assurance Process Execution and Assurance Process Reporting / Communication; detailed states/transitions remain unresolved.
Assurance Business Services and consumers	**Resolved:** Request Service Assurance for the five approved Roles; Request Service Assurance Status exclusively for System Steward; Communicate Service Assurance Outcome for applicable authorised recipients from the five Roles.
Assurance Business Interactions and participants	**Resolved:** Service Assurance Request, Service Assurance Status and Service Assurance Outcome Communication, with Service Guardian and the approved requesting/status/recipient Roles.
Assurance status versus outcome	**Resolved:** status concerns business progression only and exposes no unadjudicated finding/conclusion. Established-outcome communication remains separate and is not mandatory broadcast or responsibility for recipient action.
Financial Governance scope	**Deliberately excluded:** no distinct Harmonia financial-governance Role, Function, Capability, Service, Process or Collaboration. Financial relevance may justify authorised external receipt without responsibility transfer; exact external recipients remain unresolved.
Assurance recursion	**Resolved boundary:** Governed Assurance does not recursively assure its own execution; execution integrity belongs to the established activity execution framework without solution allocation.

Capability Tier, complete ancestry, contextual-view placement, Feature placement and structural Canonical IDs remain unresolved. Assurance Definition's formal information modelling remains deferred; documenting its conceptual output does not resolve those decisions. These bounded approvals do not complete Business Architecture or freeze/refreeze Domain03.

7. Deliberate Scope Boundaries

7.1 Financial Governance Exclusion

> **Financial Governance Exclusion** — Service Assurance findings may contain information that has financial relevance or may be consumed by external financial-governance activities. Harmonia does not establish a distinct Service Governance (Financials) Role, Function, Process or responsibility because financial governance is outside the presently established Harmonia business scope. Where financially relevant assurance outcomes are produced, they may be communicated to an appropriately authorised external recipient without Harmonia assuming financial-governance responsibility.

This is a deliberate scope exclusion, not an unresolved architectural gap. No Financial Governance Role, Function, Capability, Service, Process or Collaboration is created for symmetry. The allowance for authorised external receipt establishes no additional consumer Role or allocation in the approved Service/Interaction catalogues; exact external recipients and their mandates remain unresolved. Existing business responsibilities are preserved.

7.2 Assurance Non-Recursion Boundary

> **Governed Assurance does not recursively assure its own execution. Assurance activity execution integrity is provided by the established activity execution framework.**

This approved boundary distinguishes assurance execution integrity from independent evaluation of a governed subject. Managed execution/progression of an assurance activity does not make that execution another Service Assurance subject. No Assurance of Assurance, Guardian-of-Guardian, recursive Process, Function, Service or conclusion is introduced. System Steward's progression observation through Service Assurance Status is stewardship / management monitoring, not recursive assurance. The activity execution framework reference is solely a Business Architecture boundary and establishes no application/runtime responsibility allocation or implementation pattern.

8. Bounded Outcome and Navigation

The two assurance Roles and exactly five Functions, three Processes, three Services and three Interactions are established with approved Role relationships and capability, authority, clinical, operational and information boundaries. Further Business elements and detail remain unresolved as recorded above; Financial Governance is deliberately excluded and assurance non-recursion is established. Existing metamodel abbreviations **CT1** / **CT2** / **CT3** / **FT** and **FN** / **SV** / **PR**, established Feature identifiers and the Qualified Architectural Reference Grammar remain unchanged. No unresolved tier, ancestry, Feature placement or structural identifier is filled to accommodate assurance.

This derivation stops in Business Architecture. Information Architecture and all application, runtime, execution, representation, persistence and deployment allocation remain downstream. The completion report is execution evidence and confers no architectural authority.

- Domain03 orientation
- Capability-scoped behaviour index
- Service Assurance Modeller Role
- Service Guardian Role and boundaries
- Collaboration derivation boundary
- Approved assurance Services and consumer concerns
- Approved assurance Interactions
- Approved assurance Processes
- Financial Governance scope exclusion
- Assurance non-recursion boundary

45. Entity Management — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/01-entity-management.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Entity Management — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Entity Management establishes the canonical master data governance, identity correlation, and structural definitions for all core business entities participating in healthcare delivery: Clients/Patients, Healthcare Practitioners, Healthcare Organisations, Locations, Health Services, and Clinical Devices.

2. Client Administration

Client Administration governs the identity, demographic context, relationship associations, and privacy directives of persons receiving care.

2.1 Person Identity

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Key Architectural Semantics**:
  - **Identifier Resolution**: Asks for authoritative detail and verification regarding a specific person identifier within a single domain namespace.
  - **Identifier Correlation**: Ingests and maintains cross-authority associations between identifier namespaces where the governing relationship is established by authoritative information.
  - **Identity Alias**: Contextual, legal, or alternative identity expressions (e.g., preferred names, maiden names, emergency aliases) by which a person is known. It is conceptually distinct from cross-domain correlation.
  - **R1.x/R2.x authority boundary**: Harmonia ingests and maintains authoritative identifiers, aliases and identity relationships, processes externally authoritative corrections/merge outcomes and propagates managed-state consequences. It does not originate probabilistic matching, golden-record determination, authoritative master-person selection or person-merge decisions. REQ-FND-003 and the Strategy scope statement govern this boundary.
- **Functions & Exposed Services**:
  - **Feature: Identifier Resolution**:
    - *Function*: `ResolvePersonIdentifier` — Validates and retrieves the authoritative demographic record bound to a specific identifier.
    - *Exposed Service*: `PersonIdentifierResolution` — Exposed to Service Administration and Service Delivery to disambiguate client identities.

- **Feature: Cross-Authority Identifier Correlation**:
  - *Function*: `CorrelatePersonIdentifiers` — Correlates local identifiers (e.g., Hospital MRN, National IHI, Pathology ID) using relationships established by authoritative information, retaining their source and namespace.
  - *Exposed Service*: `PersonIdentifierCorrelation` — Exposed across the HIE to establish patient cross-reference mappings.
- **Feature: Identity Alias Association**:
  - *Function*: `MaintainPersonIdentityAliases` — Records and qualifies alias names, titles, and emergency aliases for a governed person identity.
  - *Exposed Service*: *(Internal Function — accessed via Person Identifier Resolution)*.
- **Feature: Governed Identity Correction**:
  - *Function*: `ApplyGovernedPersonIdentityCorrection` — Processes externally authoritative corrections and merge/unlink outcomes, updates governed identity associations and propagates resulting managed-state consequences with audit history. Harmonia does not adjudicate the originating person-merge decision.
  - *Exposed Service*: `PersonIdentityCorrection` — Exposed to authorized HIM/Medical Records administrators.
  - *Governed Process*: **Governed Person Identity Correction Process**.
- **Information Responsibility**: Person Identity Record, Identifier Namespace Bindings, Cross-Authority Correlation Graph, Identity Aliases, Identity Merge/Split Audit Log.

2.2 Healthcare Subject

- **Owning Capability**: `HealthcareSubject`.
- **Owner derivation**: Strategy Healthcare Subject owns Subject Context Binding and Demographic Context Governance; the Functions below discharge those same responsibilities. “Healthcare Subject Context” describes information/behaviour context, not another Capability or alias.
- **Functions & Exposed Services**:
  - **Feature: Subject Context Binding**:
    - *Function*: `EstablishHealthcareSubjectContext` — Binds demographic, cultural, and communication requirements to a patient record.
    - *Exposed Service*: `HealthcareSubjectContextResolution` — Provides comprehensive demographic, language, and indigenous status context to clinical applications.
  - **Feature: Demographic Context Governance**:
    - *Function*: `GovernSubjectDemographicContext` — Applies validation rules to address changes, date of birth verification, and death notifications.
    - *Exposed Service*: `DemographicChangeDistribution` — Emits governed subject demographic updates to subscribed clinical systems.
- **Information Responsibility**: Healthcare Subject Profile, Demographic History, Communication Preferences.

2.3 Client Relationship

- **Owning Capability**: `ClientRelationship`.
- **Owner derivation**: Strategy Client Relationship owns Next-of-Kin & Guardian Association and Relationship Validity Tracking. Support-network wording describes those responsibilities without establishing a separate Capability, alias or specialisation.
- **Functions & Exposed Services**:
  - **Feature: Next-of-Kin & Guardian Association**:
    - *Function*: `MaintainClientRelationships` — Records links between patients and carers, next of kin, nominated representatives, and legal guardians.
    - *Exposed Service*: `ClientRelationshipQuery` — Discloses verified support network members and contact details to clinical teams.

- **Feature: Relationship Validity Tracking**:
  - *Function*: `GovernClientRelationshipValidity` — Verifies power-of-attorney documents, guardianship orders, and proxy validity periods.
  - *Exposed Service*: `RepresentativeAuthorityVerification` — Confirms legal decision-making authority for substitute consent.
- **Information Responsibility**: Client Relationship Record, Representative Legal Mandate, Carer Contact Directory.

2.4 Client Privacy

- **Owning Capability**: `ClientPrivacy`.
- **Owner derivation**: Strategy Client Privacy owns Consent Directive Evaluation and Confidentiality Flag Enforcement. Consent-directive wording describes that responsibility without creating a separate Capability.
- **Functions & Exposed Services**:
  - **Feature: Consent Directive Evaluation**:
    - *Function*: `EvaluateClientConsent` — Evaluates patient opt-in, opt-out, and general information sharing preferences against proposed exchanges.
    - *Exposed Service*: `ConsentDecisionService` — Provides real-time consent evaluation decisions to Health Information Control.
  - **Feature: Confidentiality Flag Enforcement**:
    - *Function*: `EvaluateClientConfidentialityRestrictions` — Evaluates sensitive record flags (e.g., VIP masking, restricted department exclusions).
    - *Exposed Service*: `ConfidentialityRestrictionEvaluation` — Enforces record-level masking directives across the HIE.
- **Information Responsibility**: Client Consent Directive, Record Masking Rule, Disclosure Restriction Register.

3. Provider Administration

Provider Administration governs individual healthcare practitioners, their professional qualifications, clinical roles, electronic communication endpoints, and organizational affiliations.

- **Owning Capability**: `ProviderAdministration`
- **Key Architectural Semantics**:
  - **Practitioner $\neq$ Service Provider**: A *Practitioner* is an individual professional. A *Service Provider* is an organisation or facility offering health services. A Practitioner may act in the *Service Provider* role in private practice.
  - **Role and privilege**: Practitioner Role Resolution supplies role/capacity information; it does not establish Clinical Privilege or Operational Privilege. Clinical Privilege Verification retains its clinical scope. An activity may require both distinct privilege determinations; neither follows from Role information alone.
- **Functions & Exposed Services**:
  - **Feature: National Practitioner Verification**:
    - *Function*: `VerifyPractitionerRegistration` — Validates national registration, specialty endorsements, and disciplinary sanctions against regulatory registries (e.g., AHPRA).
    - *Exposed Service*: `PractitionerVerificationService` — Discloses verified registration and status to credentialing and clinical systems.
    - *Governed Process*: **Practitioner Verification Process**.
  - **Feature: Practitioner Profile Governance**:
    - *Function*: `GovernPractitionerProfile` — Maintains official names, professional identifiers (e.g., HPI-I, Prescriber Number), and clinical qualifications.
    - *Exposed Service*: `PractitionerDirectoryQuery` — Provides provider directory lookups across the healthcare network.

- **Feature: Role & Specialty Modeling**:
  - *Function*: `MaintainPractitionerRoles` — Assigns and maintains clinical roles (e.g., Specialist Physician, Registrar, General Practitioner) fulfilled by a practitioner.
  - *Exposed Service*: `PractitionerRoleResolution` — Resolves the specific clinical capacities of a practitioner.
- **Feature: Electronic Endpoint Binding**:
  - *Function*: `BindPractitionerElectronicEndpoints` — Associates secure messaging addresses, direct communication endpoints, and notification preferences to practitioner roles.
  - *Exposed Service*: `PractitionerEndpointLookup` — Provides secure message routing destinations for clinical communication.
- **Feature: Practitioner-Organisation Affiliation**:
  - *Function*: `MaintainPractitionerAffiliations` — Tracks employment, admitting rights, and consulting appointments at specific healthcare facilities.
  - *Exposed Service*: `PractitionerAffiliationResolution` — Discloses organizational ties and practice locations.
- **Feature: Clinical Privileging Record**:
  - *Function*: `MaintainClinicalPrivileges` — Records approved clinical scopes of practice, procedural authorizations, and prescribing privileges.
  - *Exposed Service*: `ClinicalPrivilegeVerification` — Verifies procedural and prescribing authority for clinical ordering systems.
- **Information Responsibility**: Practitioner Master Record, Professional Registration Status, Practitioner Role Bindings, Electronic Communication Endpoints, Scope of Practice Privileges.

4. Organisation Administration

Organisation Administration maintains the authoritative registry of healthcare organisations, networks, directorates, and administrative subdivisions.

- **Owning Capability**: `OrganisationAdministration`
- **Functions & Exposed Services**:
  - **Feature: National Organisation Verification**:
    - *Function*: `VerifyHealthcareOrganisation` — Validates enterprise legal identity, national facility identifiers (e.g., HPI-O), and accreditation status.
    - *Exposed Service*: `OrganisationVerificationService` — Provides verified organisation credentials to external and internal partners.
  - **Feature: Organisation Profile Management**:
    - *Function*: `GovernOrganisationProfile` — Maintains official trading names, enterprise registration numbers, and governing bodies.
    - *Exposed Service*: `OrganisationDirectoryQuery` — Discloses healthcare organisation profiles across the region.
  - **Feature: Hierarchical Structure Navigation**:
    - *Function*: `MaintainOrganisationalStructure` — Models parent-subsidiary relationships, clinical directorates, departments, and operational divisions.
    - *Exposed Service*: `OrganisationalHierarchyResolution` — Resolves department nesting and reporting hierarchies.
  - **Feature: Directory Contact Management**:
    - *Function*: `MaintainOrganisationContacts` — Manages administrative, clinical, and emergency contact details for facilities.
    - *Exposed Service*: `OrganisationContactLookup` — Provides official operational contact details.
- **Information Responsibility**: Healthcare Organisation Registry, National Facility Identifier Bindings, Department Hierarchy Graph, Organisation Contact Directory.

5. Location Administration

Location Administration defines the physical, functional, and geospatial structure of healthcare environments, including sites, buildings, floors, wards, rooms, bays, and bed care-places.

- **Owning Capability**: `LocationAdministration`
- **Key Architectural Semantics**:
  - **Definition vs. Operational State**: *Location Administration* defines what a physical location or care-place **IS** (its static attributes, type, capacity, hierarchy). *Health Service Operations* owns what is **operationally happening** to that care-place (occupancy, cleaning status, isolation locks, turnover).
- **Functions & Exposed Services**:
  - **Feature: Location Geospatial & Address Resolution**:
    - *Function*: `MaintainHealthcareLocation` — Records physical sites, campuses, outpatient clinics, and community health centres.
    - *Exposed Service*: `LocationDirectoryQuery` — Exposes location profiles and geospatial addresses.
  - **Feature: Physical Hierarchy Modeling**:
    - *Function*: `MaintainPhysicalLocationHierarchy` — Models the nested structural containment tree: `Campus` → `Building` → `Floor` → `Ward/Suite` → `Room/Bay` → `BedCare-Place`.
    - *Exposed Service*: `LocationHierarchyResolution` — Discloses physical containment and navigation paths for clinical facilities.
  - **Feature: Atomic Care-Place Modeling**:
    - *Function*: `MaintainCare-PlaceDefinition` — Defines static bed bays, treatment chairs, surgical tables, and emergency resuscitation cubicles, including equipment capabilities and oxygen/telemetry infrastructure.
    - *Exposed Service*: `Care-PlaceSpecificationLookup` — Discloses care-place physical capabilities to Bed & Care-Place Management.
- **Information Responsibility**: Location Master Registry, Physical Facility Hierarchy Tree, Care-Place Structural Definitions.

6. Health Service Administration

Health Service Administration governs the catalogue of clinical services offered across the healthcare network and resolves the critical multi-dimensional mapping between Services, Providers, Locations, and Practitioners.

- **Owning Capability**: `HealthServiceAdministration`
- **Key Architectural Semantics**:
  - **Service / Location / Provider Map**: This capability models the multi-way relationship: Healthcare Service ↔ Service Provider ↔ Healthcare Location ↔ Practitioner Role
  - **Ownership Invariant**: The mapping capability owns the **relationships and association rules**. It does **not** acquire ownership of the Service, Provider, Location, or Practitioner entities being mapped.
  - **Consumer Resolution Scenarios**: Enables clinical and administrative consumers to unambiguously resolve:
    1. *Who provides Service X?*
    2. *Where does Provider Y provide Service X?*
    3. *What Services are delivered at Location Z?*
    4. *Where can Service X be obtained?*
    5. *Which Practitioner Roles participate in Service X?*
- **Functions & Exposed Services**:
  - **Feature: Service Catalogue Publishing**:
    - *Function*: `MaintainHealthcareServiceDefinition` — Defines clinical service offerings (e.g., Cardiology Outpatient Clinic, MRI Imaging, Acute Stroke Care), specialty classifications, and clinical scope.

    - *Exposed Service*: `HealthcareServiceDirectoryQuery` — Exposes published clinical service offerings.
  - **Feature: Service-Location-Provider Binding**:
    - *Function*: `MaintainService/Location/ProviderMap` — Binds health service definitions to delivering organisations, physical clinic locations, and participating practitioner roles.
    - *Exposed Service*: `ServiceProvisionResolution` — Exposes multi-dimensional resolution queries answering who, where, and when healthcare services are delivered.
  - **Feature: Operating Schedule Specification**:
    - *Function*: `MaintainServiceAvailability` — Governs regular operating hours, emergency intake availability, and seasonal clinic schedules.
    - *Exposed Service*: `ServiceAvailabilityQuery` — Discloses operational intake hours and open status.
  - **Feature: Eligibility & Catchment Specification**:
    - *Function*: `MaintainServiceEligibilityRules` — Configures catchment criteria, age thresholds, referral requirements, and clinical indications for service access.
    - *Exposed Service*: `ServiceEligibilityEvaluation` — Evaluates referral eligibility for intake coordinators.
- **Information Responsibility**: Healthcare Service Catalogue, Service / Location / Provider Mapping Matrix, Service Availability Schedules, Service Eligibility Rule Sets.

7. Clinical Device Administration

Clinical Device Administration governs the master definition, instance identification, and electronic communication endpoints of medical equipment and point-of-care appliances.

- **Owning Capability**: `ClinicalDeviceAdministration`
- **Functions & Exposed Services**:
  - **Feature: Device Type Specification**:
    - *Function*: `MaintainDeviceDefinition` — Models medical device models, manufacturers, regulatory classifications (e.g., TGA/FDA classes), and calibration specifications.
    - *Exposed Service*: `DeviceDefinitionQuery` — Discloses approved medical device models and technical parameters.
  - **Feature: Device Serial Tracking**:
    - *Function*: `MaintainDeviceInstanceIdentity` — Tracks physical serial numbers, Unique Device Identifiers (UDI), asset tags, and firmware versions.
    - *Exposed Service*: `DeviceInstanceResolution` — Resolves physical device instances from scanned barcodes or UDIs.
  - **Feature: Device-Patient/Location Assignment**:
    - *Function*: `MaintainDeviceAssociation` — Records active associations between clinical devices, patient encounters, and specific care-places (e.g., Bed 4 Vital Signs Monitor).
    - *Exposed Service*: `DeviceAssociationQuery` — Discloses active device-to-patient and device-to-location bindings.
  - **Feature: Device Endpoint Registration**:
    - *Function*: `MaintainDeviceCommunicationEndpoints` — Manages network telemetry addresses, gateway ports, and protocol bindings for medical devices.
    - *Exposed Service*: `DeviceEndpointResolution` — Exposes network addresses for telemetry routing.
- **Information Responsibility**: Medical Device Type Registry, Physical Device Instance Directory, Device Association Ledger, Device Communication Endpoint Map.

46. Service Administration — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/02-service-administration.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Service Administration — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Service Administration governs the clinical administrative lifecycles, coordination workflows, and documentation exchanges that surround patient care episodes. It coordinates referrals, appointments, encounters, orders, diagnostic reports, medication events, care plans, and clinical documents across care boundaries.

2. Client-Centred Administration

- **Owning Capability**: `Client-CentredAdministration`
- **Key Architectural Semantics**:
  - **Administrative Coordination**: Handles operational demographic intake and administrative updates across departmental boundaries.
  - **Ownership Invariant**: Does **not** own authoritative person identity or identifier correlation (which is owned by *Client Administration / Person Identity*).
- **Functions & Exposed Services**:
  - **Feature: Demographic Update Ingestion**:
    - *Function*: `ReceiveClientDemographicChange` — Ingests administrative demographic updates from PAS/registration feeds.
    - *Exposed Service*: `DemographicIntakeService` — Exposed to departmental registration systems.
  - **Feature: Demographic Change Distribution**:
    - *Function*: `DistributeGovernedDemographicChange` — Propagates verified demographic modifications to subscribed clinical subsystems.
    - *Exposed Service*: `DemographicUpdateNotification` — Emits change notifications across the HIE.
- **Information Responsibility**: Administrative Intake Ledger, Subscribed Distribution Registry.

3. Referral Administration

- **Owning Capability**: `ReferralAdministration`
- **Key Architectural Semantics**:

  - **Referral vs. Order**: A **Referral** formally requests another care participant or service provider to assess, manage or assume some degree of clinical care responsibility. Intent, disposition, responsibility actually assumed and Transfer of Care are distinct; Referral itself establishes none of acceptance, delivery, assumption or Transfer of Care, and has no universal Appointment, Encounter or Order consequence. An **Order** concerns *bounded fulfilment of a specific diagnostic or therapeutic task*.
- **Functions & Exposed Services**:
  - **Feature: Referral Document Ingestion**:
    - *Function*: `ReceiveReferral` — Validates and ingests structured electronic referrals, clinical indications, and priority categories.
    - *Exposed Service*: `ReferralSubmissionService` — Exposed to external GP practices and referring health facilities.
  - **Feature: Referral Supporting Information**:
    - *Function*: `AssembleReferralContext` — Gathers relevant clinical history, medications, and diagnostic investigations into the referral package.
    - *Exposed Service*: `ReferralContextQuery` — Provides comprehensive referral dossiers to intake triage clinicians.
  - **Feature: Referral Status & Outcome Tracking**:
    - *Function*: `ManageReferralProgression` — Tracks referral disposition in an **illustrative specialist pathway; dispositions vary**: *Submitted* → *Triaged* → *Accepted/Waitlisted* → *Scheduled* → *Discharged*.
    - *Exposed Service*: `ReferralStatus&OutcomeService` — Exposes progression milestones to referrers and patients.
    - *Governed Process*: **Referral Progression Process**.
    - *Scope qualification*: The displayed specialist pathway is illustrative, not mandatory for every Referral; acceptance, waitlisting, decline and redirection retain their contextual meanings.
- **Information Responsibility**: Referral Master Record, Triage Decision Ledger, Referral Disposition Log.

4. Scheduling Administration

- **Owning Capability**: `SchedulingAdministration`
- **Key Architectural Semantics**:
  - **Coordination vs. Authoritative Booking**: Harmonia synchronises and communicates appointment milestones; it does **not** replace the authoritative booking/scheduling engines of host PAS or departmental scheduling platforms.
- **Functions & Exposed Services**:
  - **Feature: Appointment Notification Ingestion**:
    - *Function*: `ReceiveAppointmentNotification` — Ingests appointment creation, rescheduling, cancellation, and check-in events from host PAS engines.
    - *Exposed Service*: `AppointmentNotificationIngress` — Ingests booking events from departmental schedulers.
  - **Feature: Appointment Status Synchronization**:
    - *Function*: `SynchroniseAppointmentStatus` — Projects synchronised appointment timelines to clinical dashboards and care coordinators.
    - *Exposed Service*: `AppointmentScheduleQuery` — Exposes consolidated patient appointment schedules across facilities.
- **Information Responsibility**: Appointment Event History, Schedule Synchronisation State.

5. Episode & Encounter Administration

- **Owning Capability**: `Episode&EncounterAdministration`

- **Key Architectural Semantics**:
  - **Encounter Context**: Tracks the clinical and administrative interaction between a patient and healthcare provider during an emergency, inpatient, or outpatient episode.
- **Functions & Exposed Services**:
  - **Feature: Encounter Context Creation**:
    - *Function*: `EstablishEncounterContext` — Binds patient, attending practitioner, clinical class (e.g., Inpatient, Emergency, Ambulatory), and admitting diagnosis to a unique encounter identifier.
    - *Exposed Service*: `EncounterContextResolution` — Discloses active encounter details to clinical applications.
  - **Feature: Encounter State Progression**:
    - *Function*: `ProgressEncounterState` — Coordinates encounter progression (**scoped illustration; ON_LEAVE relationship unresolved**): *Planned* → *Arrived* → *In-Progress* → *Discharged* → *Completed*.
    - *Exposed Service*: `EncounterLifecycleEventNotification` — Emits encounter transition events across the HIE.
    - *Governed Process*: **Encounter Lifecycle Process**.
    - *Scope qualification*: Process completion qualifications remain local; their correspondence with Strategy `ON_LEAVE` is unresolved. A shorter summary neither removes established states nor creates universal signing or legal-closure requirements.
  - **Feature: Encounter Bed/Location Association**:
    - *Function*: `MaintainEncounterCare-PlaceAssociation` — Binds the encounter to physical wards, rooms, and beds over time.
    - *Exposed Service*: `EncounterLocationHistoryQuery` — Discloses patient location tracking history.
- **Information Responsibility**: Encounter Master Record, Encounter State Transition Log, Encounter Care-Place Movement Ledger.

6. Order Administration

- **Owning Capability**: `OrderAdministration`
- **Key Architectural Semantics**:
  - **Closed-Loop Progression**: Tracks clinical orders through closed-loop progression. Diagnostic, pathology, imaging and procedural examples illustrate scoped behaviour and do not exclude medication orders from Order Administration responsibility.
  - **Order** ≠ **Referral**: Closed-loop execution of requested investigations without transferring longitudinal care responsibility.
- **Functions & Exposed Services**:
  - **Feature: Order Request Ingestion**:
    - *Function*: `ReceiveOrderRequest` — Ingests structured orders containing clinical indication, requesting clinician, and test specifications.
    - *Exposed Service*: `OrderRequisitionIngress` — Ingests electronic orders from clinical ordering systems.
  - **Feature: Order Destination Resolution & Routing**:
    - *Function*: `ResolveOrderDestination` — Evaluates order routing rules to determine target laboratory, imaging centre, or procedural unit.
    - *Exposed Service*: `OrderDispatchService` — Transmits orders to performing diagnostic systems.
  - **Feature: Order Closed-Loop Progression Tracking**:
    - *Function*: `ManageOrderProgression` — Tracks order state (**scoped diagnostic/procedural illustration**): *Placed* → *Received* → *Specimen Collected* → *In-Progress* → *Preliminary Result* → *Final Result* → *Closed*.
    - *Exposed Service*: `OrderStatus&TrackingQuery` — Provides real-time order tracking to ordering clinicians.

    - *Governed Process*: **Closed-Loop Order Progression Process**.
    - *Scope qualification*: Specimen, investigation and result checkpoints describe the illustrated diagnostic/procedural progression. Dispatch, receipt and acceptance remain distinct; Behaviour `Received` is not equated to a Process checkpoint without evidence. Cancellation/modification responsibility remains valid even where absent from that sequence.
  - **Feature: Order Cancellation & Modification Coordination**:
    - *Function*: `CoordinateOrderModification/Cancellation` — Handles requests to amend clinical details or cancel unexecuted orders.
    - *Exposed Service*: `OrderCancellationService` — Dispatches cancel directives to performing systems.
  - **Capability-scoped behaviour: Order Result Association** (Feature identity not established):
    - *Function*: `AssociateOrderOutcome` — Correlates returning diagnostic reports and observations back to the originating order requisition.
    - *Exposed Service*: `OrderOutcomeNotification` — Emits completion notices to ordering practitioners.
- **Information Responsibility**: Clinical Order Master Record, Closed-Loop Tracking Ledger, Order-Result Correlation Matrix.

7. Diagnostic Administration

- **Owning Capability**: `DiagnosticAdministration`
- **Key Architectural Semantics**:
  - **Result Management & Distribution**: Manages the correlation, notification, and delivery of pathology, radiology, and point-of-care results.
  - **Ownership Guardrail**: Does **not** own the clinical diagnostic interpretation or specimen analysis (which is owned by the performing diagnostic service provider/LIS).
- **Functions & Exposed Services**:
  - **Feature: Diagnostic Request Correlation**:
    - *Function*: `CorrelateDiagnosticRequest` — Matches incoming lab/radiology results to active patient encounters and order requisitions.
    - *Exposed Service*: `DiagnosticCorrelationResolution` — Discloses correlated order/result linkages.
  - **Feature: Diagnostic Report Ingestion & Binding**:
    - *Function*: `BindDiagnosticReport` — Associates structured observations, reference ranges, and abnormal flags with patient health records.
    - *Exposed Service*: `DiagnosticReportIngress` — Ingests results from external LIS/RIS systems.
  - **Feature: Diagnostic Report Distribution**:
    - *Function*: `DistributeDiagnosticReport` — Delivers preliminary, final, and corrected reports to ordering and copied practitioners.
    - *Feature scope qualification*: Strategy `FEAT-SA-17 — Diagnostic Report Distributio n` retains its finalised-report responsibility. Preliminary, corrected and addendum/ correction handling belongs to the broader Diagnostic Administration responsibility; it does not redefine that narrower Feature or confer diagnostic interpretation authority.
    - *Exposed Service*: `DiagnosticResultDeliveryService` — Emits secure result notifications.
- **Information Responsibility**: Diagnostic Request-Report Linkage Registry, Result Distribution Ledger.

8. Medication Administration

- **Owning Capability**: `MedicationAdministration`

- **Key Architectural Semantics**:
  - **Event Tracking**: Aggregates medication orders, pharmacy dispense records, and administration-event information into a coherent timeline; bedside events are one supported context.
  - **Non-Practice Guardrail**: Harmonia tracks and communicates medication events; it does **not** prescribe drugs, dispense pharmaceuticals, or perform clinical administration.
- **Functions & Exposed Services**:
  - **Feature: Medication Order Ingestion**:
    - *Function*: `ReceiveMedicationOrder` — Ingests inpatient and outpatient drug prescriptions.
    - *Exposed Service*: `MedicationOrderIngress` — Ingests prescription events.
  - **Feature: Dispense Event Tracking**:
    - *Function*: `TrackDispenseEvent` — Ingests community and hospital pharmacy dispensing records.
    - *Exposed Service*: `DispenseEventIngress` — Ingests pharmacy supply notifications.
  - **Feature: Medication Administration Record (MAR) Ingestion**:
    - *Function*: `ReceiveMedicationAdministrationEvent` — Records medication administration-event information, including nurse-administered doses, e-MAR documentation and infusion telemetry as non-exhaustive examples; scope is not limited to nursing, bedside or inpatient settings.
    - *Exposed Service*: `MedicationAdministrationIngress` — Ingests administration records.
- **Information Responsibility**: Medication Event Timeline, Prescription-Dispense-Administration Correlation Matrix.

9. Procedure Administration

- **Owning Capability**: `ProcedureAdministration`
- **Scope qualification**: Procedure information extends beyond surgery; Theatre retains its distinct surgical operational responsibility.
- **Functions & Exposed Services**:
  - **Feature: Procedure Booking Ingestion**:
    - *Function*: `ReceiveProcedureBooking/Request` — Ingests procedural and surgical requisitions.
    - *Exposed Service*: `ProcedureBookingIngress` — Receives surgical/procedural requests.
  - **Feature: Procedural Documentation Ingestion**:
    - *Function*: `ReceiveProceduralDocumentation` — Ingests operation reports, anaesthetic records, and post-procedure notes.
    - *Exposed Service*: `ProceduralDocumentationIngress` — Ingests procedural documentation, including surgical notes, into patient record streams.
- **Information Responsibility**: Procedure Request Register, Procedural Documentation Index.

10. Clinical Record Administration

- **Owning Capability**: `ClinicalRecordAdministration`
- **Key Architectural Semantics**:
  - **Document Governance**: Governs the ingestion, metadata indexing, versioning, addenda, and legal lifecycle of clinical documents (discharge summaries, specialist letters, advance care directives).
- **Functions & Exposed Services**:
  - **Feature: Clinical Document Ingestion**:
    - *Function*: `ReceiveClinicalDocument` — Receives clinical documents from information suppliers.

    - *Exposed Service*: `ClinicalDocumentIngress` — Exposes document submission endpoints across the HIE.
  - **Feature: Document Versioning & Supersession**:
    - *Function*: `GovernClinicalDocumentLifecycle` — Manages document states (**scoped illustration, not a universal transition model**): *Draft* → *Preliminary* → *Final* → *Amended* → *Superseded* → *Entered-in-Error*.
    - *Exposed Service*: `ClinicalDocumentLifecycleService` — Allows authoring clinicians to publish addenda or corrections.
    - *Governed Process*: **Clinical Document Lifecycle Process**.
    - *Scope qualification*: `Final` $\neq$ `automatically Final Signed`. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Process signing/countersigning requirements retain their local scope, and amendment/supersession/error states are not mandatory for every document. Ingestion and consumer processing do not define originating validity, authority, authorship or legal status.
  - **Feature: Document Metadata Indexing**:
    - *Function*: `IndexClinicalDocumentMetadata` — Indexes author, specialty, encounter, date, and document type (LOINC/SNOMED).
    - *Exposed Service*: `DocumentMetadataRegistryQuery` — Discloses document registry metadata to clinical viewers.
- **Information Responsibility**: Clinical Document Registry, Document Version History, Document Lifecycle State Register.

11. Care Plan Administration

- **Owning Capability**: `CarePlanAdministration`
- **Functions & Exposed Services**:
  - **Feature: Care Plan Ingestion & Distribution**:
    - *Function*: `ReceiveCarePlan` — Ingests multidisciplinary care plans, clinical goals, and patient action steps.
    - *Exposed Service*: `CarePlanIngress` — Ingests shared care plans.
  - **Capability-scoped behaviour: Care Plan Context Association** (Feature identity not established):
    - *Function*: `AssociateCarePlanContext` — Links care plans to active diagnoses, health problems, and care team members.
    - *Exposed Service*: `CarePlanContextQuery` — Discloses active care plans to treating clinicians.
  - **Capability-scoped behaviour: Care Plan Update Distribution** (Feature identity not established):
    - *Function*: `DistributeCarePlanChange` — Broadcasts care plan updates to all enrolled care team participants.
    - *Exposed Service*: `CarePlanChangeNotification` — Emits care plan revision notices.
- **Information Responsibility**: Shared Care Plan Registry, Clinical Goal Ledger, Care Plan Revision History.

12. Clinical Communication Administration

- **Owning Capability**: `ClinicalCommunicationAdministration`
- **Key Architectural Semantics**:
  - **Technical vs. Business Acknowledgement**:
    - **Technical ACK**: May establish durable receiving-boundary acceptance into an integration queue; it does not establish business acceptance.

    - **Business Delivery ACK**: Records contextual recipient-reported facts or dispositions, which may include receipt, parsing, acceptance, rejection, filing or another explicit disposition. These are not a universal progression; business-acceptance and filing criteria remain unresolved.
- **Functions & Exposed Services**:
  - **Feature: Clinical Communication Distribution**:
    - *Function*: `DistributeClinicalCommunication` — Routes secure messages, referral letters, and critical alerts to practitioner endpoints.
    - *Exposed Service*: `SecureClinicalMessageDispatch` — Exposes message dispatch to clinical authors.
  - **Feature: Delivery Acknowledgement Tracking**:
    - *Function*: `TrackBusinessDeliveryAcknowledgement` — Tracks end-to-end delivery state and records business-level delivery/rejection acknowledgements.
    - *Receipt qualification*: Delivery/read evidence does not establish clinical review, agreement, approval or incorporation. Clinical Review itself does not automatically establish incorporation; no universal Clinical Incorporation concept is established.
    - *Exposed Service*: `CommunicationDeliveryStatusQuery` — Discloses delivery confirmation to message senders.
  - **Feature: Communication Audit Logging**:
    - *Function*: `RecordCommunicationTransaction` — Maintains an immutable audit trail of clinical message dispatch and receipt.
    - *Exposed Service*: `CommunicationAuditQuery` — Discloses transmission history for medico-legal verification.
- **Information Responsibility**: Clinical Message Dispatch Log, Business Delivery Acknowledgement Ledger, Medico-Legal Transmission Audit Register.
- **Authority boundary**: Acknowledgement, receipt, persistence, filing, indexing, assembly and review do not transfer originating authority or establish finality. Communication audit evidence concerns transmission; it does not establish the clinical content's authority or legal qualification.

47. Service Delivery — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/03-service-delivery.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Service Delivery — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Service Delivery represents the clinical operating contexts in which direct patient care is delivered across primary, acute, emergency, inpatient, diagnostic, and virtual settings.

The Fundamental Care Enablement Principle

> **Harmonia enables, informs, and coordinates healthcare delivery across care boundaries; it does not practice medicine, prescribe pharmaceuticals, make autonomous clinical decisions, or replace clinician judgment.**

Service Delivery capabilities within Harmonia define **integration enablement and clinical context correlation behaviour**. They reuse and consume core capabilities (*Patient Clinical Record, Health Information Exchange, Health Information Control, Episode & Encounter Administration, Clinical Record Administration*) rather than duplicating their responsibilities.

Residual Feature Sufficiency

The 2026-10-09 human adjudications apply the Business composition and clinical-work boundaries. General clinical-information capture, processing, preservation and distribution are expressed by Patient Clinical Record and Health Information Exchange, with applicable Service Administration responsibility. A clinical setting does not establish specialised capture or clinical-work ownership.

Strategy Feature	Adjudicated Business expression and boundary
`FEAT-SD-03 — Acute Clinical State Event Capture`	Acute deterioration/status information is managed through general clinical-information responsibilities. No Acute-specific capture Function is required. The Strategy Feature's acute-specific framing is over-specialised if interpreted as a separate Harmonia responsibility; its existing name/definition are retained for bounded Strategy review, not silently redefined. Alert routing below remains distinct from capture.

Strategy Feature	Adjudicated Business expression and boundary
`FEAT-SD-08 — Inpatient Clinical Progression Tracking`	Relevant milestone information uses the same general responsibilities. Operational consequences may inform patient flow, capacity or discharge coordination; those consumers do not establish the originating clinical fact. An allied-health clearance may affect discharge readiness without becoming an operationally authored clearance. No Inpatient-specific clinical-tracking Function or EMR workflow is required. Dossier assembly below remains distinct.
`FEAT-SD-14 — Screening Recall Notification Distribution`	Recall is a domain-specific composition of clinical-information management, Calendar Management, Workflow & Activity Coordination and information communication/distribution, with applicable entity coordination. Configured workflow/Praxis may realise it without a screening-specific recall Function. Eligibility projection alone remains distinct from distribution. Exact workflow and provider contracts are downstream, not established by this sufficiency decision.

These are composition-level sufficiency decisions, not replacement Feature associations to the Functions below. The same reusable responsibilities can support follow-up, vaccination, diagnostic or review reminders without a new Business Function for each clinical purpose. Population/cohort identification may precede individual activity. Once activity concerns a specific managed patient, the entity-specific Digital Twin coordination rule applies; it establishes no Domain03 Twin Actor, Process or execution mechanism.

2. Clinical Care Enablement Contexts

RAD source listing (presentation only; no inferred ancestry).

Service Delivery Contexts

Primary Care

Acute Care

Emergency Care

Inpatient Care

Diagnostic Services

Medication Therapy

Preventive Care

Community Care

Outreach Care

Remote & Virtual Care

2.1 Primary Care

- **Owning Capability**: `PrimaryCare`
- **Clinical Operating Context**: General practice clinics, community medical centres, and family medicine practices providing longitudinal whole-person care.
- **Enabling Functions & Exposed Services**:
  - **Feature: General Practice Shared Record Ingestion**:
    - *Function*: `CorrelatePrimaryCareConsultationContext` — Binds GP consultation notes, vital signs, and encounter episodes to the regional Longitudinal Health Record (LHR).
    - *Exposed Service*: `PrimaryCareSummaryIngress` — Ingests GP event summaries into the HIE.
  - **Capability-scoped Functions / Services**:

  - *Feature association not established*: Strategy `FEAT-SD-02 — Primary Care Notification Dispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
  - *Function*: `CoordinateMulti-AgencyChronicCarePlan` — Enables GPs to track multi-provider disease management milestones (e.g., Diabetes or Cardiovascular Care Plans).
  - *Exposed Service*: `ChronicCareTimelineQuery` — Exposes aggregated multidisciplinary care timelines.
- **Typed architectural dependencies**:
  - **Service consumption**: `PersonIdentifierResolution`; `LongitudinalClinicalRecordQuery` from Patient Clinical Record, already established in the consumption matrix.
  - **Capability dependency**: `CarePlanAdministration`.
  - **Unresolved reference**: “Referral Submission” — an exact Service identity/consuming relationship is not established here; the abbreviated wording is not an alias.

2.2 Acute Care

- **Owning Capability**: `AcuteCare`
- **Clinical Operating Context**: Hospital specialty wards, surgical suites, and intensive care units managing severe, urgent, or post-operative conditions.
- **Enabling Functions & Exposed Services**:
  - **Feature: Acute Care Clinical Context Provision**:
    - *Function*: `AssembleAcuteClinicalBaseline` — Rapidly aggregates previous discharge summaries, active medications, allergies, and diagnostic alerts upon acute hospital presentation.
    - *Exposed Service*: `AcuteAdmissionDossierQuery` — Delivers a high-priority clinical briefing to admitting teams.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-SD-03 — Acute Clinical State Event Capture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.
    - *Function*: `RouteAcuteCriticalResultAlert` — Immediately directs urgent lab/radiology alerts (e.g., critical troponin, intracranial bleed) to active acute attending clinicians.
    - *Exposed Service*: `AcuteClinicalAlertDispatch` — Dispatches urgent interruptive alerts to on-duty ward teams.
- **Typed architectural dependencies**:
  - **Service consumption**: `EncounterContextResolution`.
  - **Capability dependency**: `HealthInformationControl`; break-glass access describes the required governed context, not an independently allocated Service.
  - **Unresolved reference**: “Diagnostic Result Delivery” — intended exact Service/relationship remains unestablished; no alias or replacement edge is inferred.

2.3 Emergency Care

- **Owning Capability**: `EmergencyCare`
- **Clinical Operating Context**: Emergency Departments (ED), trauma centres, and urgent care clinics operating under time-critical, high-acuity constraints.
- **Enabling Functions & Exposed Services**:
  - **Feature: Emergency Department Arrival & Triage Ingestion**:
    - *Function*: `BindEDTriageAssessment` — Captures ATS (Australasian Triage Scale) category, presenting complaint, and ambulance paramedic handover data.
    - *Exposed Service*: `EDTriageStreamIngress` — Ingests emergency triage presentations.
  - **Feature: Emergency Care Cross-Facility Correlation**:

    - *Function*: `ProvideEmergencyLHRSummary` — Delivers rapid, override-capable access to Advance Care Directives, allergy records, and recent hospital admissions.
    - *Exposed Service*: `EmergencyFast-PathLHRQuery` — Provides low-latency emergency record summaries.
- **Typed architectural dependencies**:
    - **Service consumption**: `PersonIdentifierResolution`. "Fast-Match" does not establish a separate Service or probabilistic/master-person matching behaviour.
    - **Capability dependency**: `ClientPrivacy` for its established consent/privacy responsibility; "Break-Glass Override" does not allocate override authority or another element.
    - **Unresolved relationship to an established Feature**: `FEAT-SA-09 — Encounter State Progression`. A precise Service-consumption relationship is not established by naming that Feature.

2.4 Inpatient Care

- **Owning Capability**: `InpatientCare`
- **Clinical Operating Context**: Inpatient hospital wards, rehabilitation units, and sub-acute facilities managing multi-day hospital stays.
- **Enabling Functions & Exposed Services**:
    - **Feature: Inpatient Rounding Context Provision**:
        - *Function*: `ConsolidateInpatientClinicalRoundFeed` — Aggregates daily vital signs, overnight pathology, allied health notes, and medication administration records.
        - *Exposed Service*: `WardRoundClinicalBriefingQuery` — Discloses consolidated 24-hour clinical briefing views.
    - **Capability-scoped Functions / Services**:
        - *Feature association not established*: `FEAT-SD-08 — Inpatient Clinical Progression Tracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.
        - *Function*: `AssembleInpatientDischargeDossier` — Synthesises inpatient diagnoses, procedural summaries, reconciled discharge medications, and follow-up instructions into a canonical discharge summary package.
        - *Exposed Service*: `DischargeDossierPackagingService` — Emits canonical discharge summaries to GPs and community providers.
- **Typed architectural dependencies**:
    - **Capability dependency**: `DischargeManagement`.
    - **Unresolved references / relationships**: "Encounter Care-Place Movement," "Clinical Document Lifecycle," "Medication Administration Event Ingestion." Their intended element types and exact consumption/participation relationships are not established by these descriptions; no Function, Process or Service equivalence is inferred.

2.5 Diagnostic Services

- **Owning Capability**: `DiagnosticServices`
- **Clinical Operating Context**: Clinical pathology laboratories, diagnostic imaging networks (radiology/MRI/CT), nuclear medicine, and clinical neurophysiology departments.
- **Enabling Functions & Exposed Services**:
    - **Capability-scoped Functions / Services**:
        - *Feature association not established*: Strategy `FEAT-SD-09 — Laboratory & Imaging Result Distribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
        - *Function*: `ReceiveDiagnosticRequisitionFeed` — Normalises order requisitions from across the health network into modality worklists.

    - *Exposed Service*: `ModalityWorklistDistribution` — Supplies order worklists to diagnostic laboratory/imaging systems.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-10 – Diagnostic History Consolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `PublishGovernedDiagnosticReport` — Normalises and distributes structured diagnostic observations, reference ranges, and diagnostic image study links into the LHR.
    - *Exposed Service*: `DiagnosticPublicationService` — Publishes verified diagnostic reports across the enterprise.
- **Capability dependencies**: `OrderAdministration` for closed-loop order context; `InformationDesignGovernance` for governed terminology bindings. These contextual purposes establish neither an additional Feature nor a precise Service-consumption edge.

2.6 Medication Therapy

- **Owning Capability**: `MedicationTherapy`
- **Clinical Operating Context**: Hospital pharmacies, clinical pharmacotherapy review panels, community dispensing networks, and outpatient infusion suites.
- **Enabling Functions & Exposed Services**:
  - **Feature: Medication History Reconciliation Ingestion**:
    - *Function*: `ProjectLongitudinalMedicationHistory` — Aggregates community dispensing, hospital e-MAR administrations, and specialist prescriptions into a unified chronological medication profile.
    - *Exposed Service*: `LongitudinalMedicationHistoryQuery` — Discloses full medication timelines for medication reconciliation.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-12 – Medication Administration Tracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `CorrelateAdverseReaction&AllergyProfile` — Binds confirmed substance allergies, adverse drug reactions, and severity grades to the patient identity.
    - *Exposed Service*: `Allergy&AdverseReactionQuery` — Exposes active allergy profiles to clinical decision support systems.
- **Capability dependencies**: `MedicationAdministration`, `PatientClinicalRecord`, `HealthInformationControl`; no specific Service is allocated by this contextual list.

2.7 Preventive Care

- **Owning Capability**: `PreventiveCare`
- **Clinical Operating Context**: Public health screening programmes (breast/bowel/cervical), national immunisation registries, and health promotion initiatives.
- **Enabling Functions & Exposed Services**:
  - **Feature: Immunisation Event Submission**:
    - *Function*: `SyndicateImmunisationRecord` — Records vaccine encounters and syndicates records to national immunisation registers (e.g., AIR).
    - *Exposed Service*: `ImmunisationHistoryQuery` — Discloses complete vaccination history.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-SD-14 – Screening Recall Notification Distribution` is adjudicated sufficient through reusable composition/workflow above.

Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.
    - *Function*: `ProjectScreeningEligibilityTimeline` — Tracks age-based and risk-based screening schedules across primary care.
    - *Exposed Service*: `PreventiveScreeningStatusQuery` — Discloses screening due dates to primary care providers.
- **Typed architectural dependencies**:
    - **Capability dependency**: `HealthInformationExchange` for syndication.
    - **Unresolved reference**: "Healthcare Subject Demographic Context" — exact element/relationship remains unestablished; descriptive context does not create a Capability alias or Service.

2.8 Community Care

- **Owning Capability**: `CommunityCare`
- **Clinical Operating Context**: District nursing, home-based palliative care, community mental health teams, and allied health community outreach.
- **Enabling Functions & Exposed Services**:
    - **Feature: Community Encounter Ingestion**:
        - *Function*: `SynchronizeCommunityVisitRecord` — Ingests clinical notes, wound assessments, and vital signs recorded in community and home settings.
        - *Exposed Service*: `CommunityCareEncounterIngress` — Ingests community care documentation.
    - **Capability-scoped Functions / Services**:
        - *Feature association not established*: Strategy `FEAT-SD-16 — Community Care Plan Synchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
        - *Function*: `BindCommunitySupportNetworkContext` — Links community nursing teams with local non-clinical social care and home care packages.
        - *Exposed Service*: `CommunityCareTeamQuery` — Exposes community provider contact rosters.
- **Typed architectural dependencies**:
    - **Capability dependencies**: `ClientRelationship`, `Episode&EncounterAdministration`.
    - **Collaboration reference with unresolved involvement**: `Care-TeamCollaboration` is an established Collaboration; precise participation/involvement is not allocated by the former "Consumes" wording.

2.9 Outreach Care

- **Owning Capability**: `OutreachCare`
- **Clinical Operating Context**: Mobile specialist clinics, rural and remote fly-in fly-out (FIFO) clinical teams, and school health screening programmes.
- **Enabling Functions & Exposed Services**:
    - **Feature: Outreach Visit Context Provision**:
        - *Function*: `ProvideOutreachVisitContext` — Packages and delivers portable clinical summaries and offline-capable care summaries for remote outreach clinics.
        - *Derivation*: `FEAT-SD-17` in Strategy's Outreach Care Continuity establishes this preparation/provision responsibility. Outcome reconciliation below does not supply pre-visit context. This Function remains owned by Outreach Care, uses governed source information without acquiring its originating authority and establishes no copying, storage or offline implementation mechanism.
        - *Exposure boundary*: Remote outreach clinics are the established recipients. No separate named Service, Process, Role allocation or structural ID is introduced merely to mirror the Function.

    - **Feature: Outreach Encounter Outcome Ingestion**:
        - *Function*: `ReconcileOutreachClinicalBatch` — Reconciles and merges clinical documentation recorded during disconnected remote clinic operations.
        - *Exposed Service*: `OutreachBatchReconciliationService` — Reconciles batch clinical encounters upon network reconnection.
- **Typed architectural dependencies**:
    - **Capability dependencies**: `HealthInformationExchange`, `ClinicalRecordAdministration`.
    - **Established Service with unresolved consumption applicability**: `PersonIdentityCorrection` retains Person Identity ownership; how Outreach Care requires that Service is not established here. It cannot authorise originating person-merge decisions.

2.10 Remote & Virtual Care

- **Owning Capability**: `Remote&VirtualCare`
- **Clinical Operating Context**: Telehealth consultations, virtual hospital-in-the-home (vHIT), remote patient monitoring (RPM), and asynchronous specialist e-consults.
- **Enabling Functions & Exposed Services**:
    - **Feature: Virtual Care Session Context Binding**:
        - *Function*: `CorrelateVirtualCareConsultationContext` — Binds video consultation metadata, remote practitioner identity, and shared digital whiteboard artifacts to the encounter.
        - *Exposed Service*: `VirtualEncounterContextResolution` — Binds virtual session identifiers to patient encounters.
    - **Feature: Remote Telemetry Ingestion**:
        - *Function*: `IngestRemotePatientTelemetry` — Ingests continuous or periodic physiological measurements (e.g., blood pressure, pulse oximetry, continuous glucose) from home monitoring devices.
        - *Exposed Service*: `RemoteTelemetryIngress` — Ingests remote patient data streams into the LHR.
- **Capability dependencies**: `ClinicalDeviceAdministration`, `PatientClinicalRecord`, `ClinicalCommunicationAdministration`; no specific Service is allocated by this contextual list.

48. Health Service Operations — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/04-health-service-operations.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Health Service Operations — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Health Service Operations governs the operational logistics, physical resource management, facility capacity, staff dispatch, and operational movement required to support healthcare delivery across clinical facilities.

Operational Logistics vs. Software Plumbing

> **In Domain 03, "Operations" refers strictly to healthcare facility logistics (e.g., bed turnover, patient portering, theatre case scheduling, clinic queues, dispatch). It does not mean IT middleware monitoring, application deployment, or database administration.**

Where Healthcare Service context materially affects operational meaning, authority, coordination, progression or accountability, keep it identifiable with the activity and relevant information responsibility. Location, Organisation, Practitioner or Role does not imply that service context. This contextual obligation establishes no information representation, identifier or implementation binding.

The bounded coverage additions below derive from the established Strategy HSO Features. They reuse existing compound behaviour where its meaning supports the relationship, or add the minimum directly required behaviour. Additional Functions do not mechanically create Services, Processes, Roles or structural IDs.

Capacity Management Responsibility Boundary

The 2026-10-09 human adjudication establishes **Capacity Management** as Business responsibility for understanding and managing available, committed and utilised capacity in six service-delivery operational contexts. Clinic/practice, ward, theatre, ED, outpatient and mobile service capacity retain their distinct Business meanings. This does not establish a common capacity model, information schema or lifecycle. Generalised service/location/patient/operational-state capture belongs to downstream derivation and is not a Domain03 Function.

Context-specific responsibility establishes/manages service capacity. Resource state contributes to it: an available bed alone does not mean a ward can accept another patient; staffing, demand, acuity and operational constraints may also matter. Service Capacity Management

consumes/coordinates context-specific capacity; Work Allocation & Dispatch and Discharge Management may consume capacity/availability or capacity/progression information without becoming its owners. These are Business responsibility relationships, not newly allocated Services or consumption contracts.

HSO context	Context-specific capacity responsibility
Clinic & Practice Operations	Yes — session/queue behaviour contributes; overall clinic/ practice capacity is expressed separately in §2.1.
Ward Operations	Yes — existing operational coordination is reframed as ward capacity in §2.2.
Theatre Operations	Yes — case progression contributes; theatre capacity is expressed separately in §2.3.
Emergency Department Operations	Yes — existing departmental capacity behaviour is clarified in §2.4.
Outpatient Operations	Yes — existing session utilisation is reframed as outpatient capacity in §2.5.
Bed & Care-Place Management	No — establishes resource state contributing to ward capacity.
Clinical Resource Management	No — establishes equipment state/availability; resource readiness is distinct from service capacity.
Service Capacity Management	No — consumes/coordinates context-specific capacity.
Mobile Staff Management	Yes — presence and visit progression contribute; mobile service capacity is expressed separately in §2.9.
On-Call Management	No — resolves coverage rather than establishing context-specific capacity.
Work Allocation & Dispatch	No — may consume capacity/availability for operational work.
Credential Management	No — verifies operational access.
Patient Transport	No — coordinates transport requests/progression.
Clinical Logistics Coordination	No — coordinates logistics/custody.
Discharge Management	No — may consume capacity/progression for discharge coordination.
Clinical Qualification Management	No — verifies qualifications/competency for applicable dispatch.

Clinical work integration does not confer clinical-work management, clinical handover, clinical task determination, clinical allocation authority or EMR worklist ownership. Capacity and progression consumers retain the originating authority of clinical facts.

2. Facility Operational Units & Queue Coordination

RAD source listing (presentation only; no inferred ancestry).

Health Service Operations Contexts

1. Clinic & Practice Operations
2. Ward Operations
3. Theatre Operations
4. Emergency Department Operations
5. Outpatient Operations
6. Bed & Care-Place Management
7. Clinical Resource Management
8. Service Capacity Management
9. Mobile Staff Management
10. On-Call Management
11. Work Allocation & Dispatch
12. Credential Management

13. Patient Transport
14. Clinical Logistics Coordination
15. Discharge Management
16. Clinical Qualification Management

2.1 Clinic & Practice Operations

- **Owning Capability**: `Clinic&PracticeOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageClinic&PracticeCapacity` — Understands and manages available, committed and utilised clinic/practice capacity in relation to practitioner availability, sessions, rooms, appointments and patient demand.
    - *Responsibility boundary*: Operating-session tracking and individual queue progression contribute to this responsibility; neither alone expresses overall service capacity. Capacity coordination does not own source bookings, clinical decisions or practitioner rostering.
  - **Feature: Clinic Session Tracking**:
    - *Function*: `TrackClinicOperatingSession` — Ingests and tracks clinic operating-session start, delay and completion, distinct from an individual patient's queue progression.
    - *Derivation*: `FEAT-HSO-01` directly establishes this session-level behaviour; Manage Clinic Queue Progression and its Process concern individual patients and do not supply it.
    - *Capacity contribution*: Session start/delay/completion remains distinct observable Business behaviour contributing to clinic/practice capacity; it is not merged into capacity merely for structural neatness.
  - **Feature: Operational Queue Progression**:
    - *Function*: `ManageClinicQueueProgression` — Tracks patient arrival, rooming, consultation in-progress, and check-out states.
    - *Exposed Service*: `ClinicQueueTelemetryService` — Discloses real-time clinic waiting times and queue states.
    - *Governed Process*: **Clinic / Operational Queue Progression Process**.
- **Information Responsibility**: Clinic & Practice Capacity, Clinic Session Progression, Clinic Queue State, Patient Waiting Time Metrics.

2.2 Ward Operations

- **Owning Capability**: `WardOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageWardCapacity` — Coordinates available, committed and utilised ward capacity and its patient-flow implications, retaining acuity distribution, nurse-to-patient ratios and isolation cohorting alongside bed/care-place state, patient demand and operational constraints.
    - *Name Alias*: `CoordinateWardOperationalState` — the same established ward operational responsibility, clarified as Capacity Management; no new Function identity or identifier is allocated.
    - *Exposed Service*: `WardOperationalSummaryQuery` — Discloses ward-level operational telemetry.
    - *Existing Feature contribution*: Ward Patient Flow Coordination concerns ward occupancy, care-place allocation and patient bed status. Those patient-flow responsibilities remain within ward operational coordination; they do not define the whole capacity responsibility.

    - *Capacity boundary*: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.
  - **Feature: Clinical Handover Context Assembly**:
    - *Function*: `AssembleWardHandoverContext` — Collates current ward patient lists, active medical concerns and pending tasks for nursing shift handover.
    - *Derivation*: `FEAT-HSO-04` directly requires this assembly. Ward acuity/cohorting telemetry alone does not provide the stated handover content. Source clinical information retains its originating authority; assembly does not perform clinical review, manage clinical handover, determine pending clinical work or acquire clinical judgement.
- **Information Responsibility**: Ward Capacity, Ward Shift Status Ledger, Acuity Balance Register, Ward Handover Context.

2.3 Theatre Operations

- **Owning Capability**: `TheatreOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageTheatreCapacity` — Understands and manages available, committed and utilised theatre capacity in relation to theatre, team and equipment availability, scheduled cases and case progression.
    - *Responsibility boundary*: Case milestones and targeted resource notifications contribute; they do not by themselves establish available or committed theatre service capacity. Clinical case determination and originating team/equipment information remain separately governed.
  - **Feature: Operating Theatre Case Progression**:
    - *Function*: `ManageTheatreCaseProgression` — Coordinates surgical case milestones: *Case Called → Patient in Anaesthetic Bay → Anaesthesia Commenced → Knife to Skin → Procedure Finished → In PACU/Recovery → Ward Handover Complete*.
    - *Exposed Service*: `TheatreCaseProgressionTelemetry` — Emits real-time surgical milestone updates.
    - *Additional Feature responsibility*: `FEAT-HSO-06 – Perioperative Resource Notificatio n` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.
    - *Governed Process*: **Theatre Case Progression Process**.
- **Information Responsibility**: Theatre Capacity, Theatre Slate Register, Perioperative Milestone Log.

2.4 Emergency Department Operations

- **Owning Capability**: `EmergencyDepartmentOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `CoordinateEDDepartmentalCapacity` — Understands and manages available, committed and utilised ED capacity in relation to care-place availability, demand, patient presence/progression, acuity and operational constraints; retains waiting room occupancy, resuscitation bay availability and time-to-triage metrics.
    - *Exposed Service*: `EDOperationalStatusQuery` — Discloses facility ED occupancy and surge level.
    - *Existing Feature scope*: ED Length of Stay Tracking retains the existing wait/progression metrics context; capacity responsibility is broader than that Feature. This clarification does not assert a new whole-Feature realisation relationship.

    - *Additional Feature relationship*: `FEAT-HSO-07 — ED Bed & Bay Tracking` is materially realised by this capacity-tracking behaviour for occupancy/status of emergency cubicles, resuscitation bays and short-stay beds. Those care-place states are explicit within the existing Function and its operational status exposure.
    - *Capacity boundary*: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.
- **Information Responsibility**: ED Capacity, ED Operational Dashboard Ledger, Resuscitation Bay Occupancy Matrix.

2.5 Outpatient Operations

- **Owning Capability**: `OutpatientOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped Functions / Services**:
    - *Function*: `ManageOutpatientCapacity` — Understands and manages available, committed and utilised outpatient capacity through clinic/session, practitioner and room availability, appointment demand, patient arrival/waiting and operational progression. Retains specialty room allocation and clinician arrival tracking.
    - *Name Alias*: `CoordinateOutpatientSessionUtilisation` — the existing responsibility reframed as outpatient Capacity Management; no new Function identity or identifier is allocated.
    - *Feature responsibility*: `FEAT-HSO-09 — Outpatient Arrival Registration` is realised within this capacity behaviour by ingesting patient check-in/arrival events and notifying treating clinicians. Patient arrival contributes demand/presence; it does not itself establish capacity. This bounded responsibility does not equate the whole capacity Function with the Feature or create a separate Arrival Registration Function.
    - *Exposed Service*: `AmbulatorySessionStatusQuery` — Exposes clinic room occupancy.
    - *Exposure qualification*: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description. That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.
    - *Capacity boundary*: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.
  - **Feature: Post-Clinic Order Coordination**:
    - *Function*: `CapturePost-ClinicFollow-upRequirements` — Captures follow-up booking and diagnostic-order requirements generated at clinic completion.
    - *Derivation*: `FEAT-HSO-10` directly establishes this capture responsibility. The Clinic queue Process's consultation-concluded checkpoint describes orders placed, not Outpatient Operations' capture of follow-up requirements. Capture does not authorise clinical orders, own authoritative bookings or establish a new Service dependency.
- **Information Responsibility**: Outpatient Capacity, Outpatient Arrival/Waiting Demand, Outpatient Session Schedule, Room Allocation Register, Post-Clinic Follow-up Requirements.

2.6 Bed & Care-Place Management

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Key Architectural Semantics**:
  - **Operational State vs. Definition**: While *Location Administration* defines the physical care-place, *Bed & Care-Place Management* owns the real-time operational status (*Available*, *Occupied*, *Reserved*, *Blocked*, *Dirty*, *Cleaning In-Progress*, *Maintenance Lock*).
  - **Resource State vs. Capacity**: This state contributes to Ward Operations' capacity responsibility. Bed availability does not establish ward service capacity or authority to accept another patient.

- **Functions & Exposed Services**:
  - **Feature: Bed Availability Tracking**:
    - *Function*: `MaintainOperationalBedState` — Tracks real-time bed status and environmental locks.
    - *Exposed Service*: `BedAvailabilityQuery` — Discloses real-time bed availability across hospital directorates.
  - **Feature: Bed State Progression**:
    - *Function*: `ManageBedTurnoverProgression` — Coordinates cleaning, disinfection, and preparation: *Vacated* → *Cleaning Requested* → *Cleaning In-Progress* → *Inspected/Ready* → *Occupied*.
    - *Exposed Service*: `BedTurnoverLifecycleService` — Coordinates turnover requests with environmental services.
    - *Additional Feature responsibility*: `FEAT-HSO-13 – Bed Readiness Notification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.
    - *Governed Process*: **Bed Turnover Process**.
- **Information Responsibility**: Operational Bed Status Registry, Care-Place Lock Register, Bed Turnover Milestone Log.

2.7 Clinical Resource Management

- **Owning Capability**: `ClinicalResourceManagement`
- **Functions & Exposed Services**:
  - **Feature: Mobile Clinical Equipment Tracking**:
    - *Function*: `TrackMobileClinicalAssetOperationalState` — Manages the real-time location and operational readiness of mobile ventilators, infusion pumps, and telemetry transmitters.
    - *Exposed Service*: `ClinicalResourceAvailabilityQuery` — Exposes mobile equipment availability.
- **Information Responsibility**: Mobile Clinical Asset Operational Ledger, Maintenance Readiness Matrix.

2.8 Service Capacity Management

- **Owning Capability**: `ServiceCapacityManagement`
- **Functions & Exposed Services**:
  - **Feature: Operational Capacity Metric Aggregation**:
    - *Function*: `AggregateServiceCapacityTelemetry` — Consumes and coordinates context-specific service-capacity information for broader situational awareness, alongside bed occupancy, ICU surge capacity and ventilator availability across regional hospitals.
    - *Exposed Service*: `RegionalHealthSystemCapacityQuery` — Provides executive operational situational awareness.
    - *Consumer boundary*: Aggregation does not establish the underlying context-specific capacity. Resource occupancy/readiness and service capacity remain distinguishable; exact additional provider contracts are unestablished.
- **Information Responsibility**: Regional System Capacity Snapshot, Surge Level State.

2.9 Mobile Staff Management

- **Owning Capability**: `MobileStaffManagement`

- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageMobileServiceCapacity` — Understands and manages available, committed and utilised mobile-service capacity in relation to workforce availability, geography, time, workload and applicable service demand.
    - *Responsibility boundary*: Staff presence and externally established assignments/visit progression contribute to capacity; they do not alone express it. Capacity coordination does not determine clinical work, own the source workforce roster or allocate underlying clinical duties.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-HSO-16 — Mobile Worker Task Dispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.
    - *Function*: `TrackOperationalStaffPresence` — Tracks active on-duty presence and physical zone assignment for porters, phlebotomists, and roving nurses.
    - *Exposed Service*: `StaffPresenceTelemetryQuery` — Discloses on-duty staff availability by hospital zone.
  - **Feature: Mobile Visit Status Tracking**:
    - *Function*: `ReceiveMobileVisitProgression` — Ingests arrival, visit-progression and safety check-in milestones from roving staff.
    - *Derivation*: `FEAT-HSO-17` directly requires visit-specific milestone ingestion. It remains distinct from presence, capacity assessment and work-management integration. Received progression can inform mobile capacity without determining clinical work or clinical completion.
  - **Feature: Mobile Worker Task Dispatch** (integration scope qualified in Strategy):
    - *Function*: `IntegrateMobileWorkManagement` — Exchanges externally established work/assignment, dispatch and operational progression/status information, with applicable worker/context information, between mobile work-management mechanisms and Harmonia operational coordination.
    - *Derivation and boundary*: The human adjudication of `FEAT-HSO-16` establishes integration with mobile work/task management. This responsibility is absent from presence observation and visit-specific milestone ingestion. It does not determine, allocate or manage the underlying clinical work, clinical handover or EMR worklists. Work Allocation & Dispatch retains non-clinical operational Work Order responsibility.
    - *Terminology*: External “work/task” vocabulary does not make human work a Harmonia Task. Work Order means human doing; To Do means human review/update/approval; Task means synthetic task. The preferred Strategy name `MobileWorkTaskManagementIntegration` remains a proposed naming correction pending human confirmation, not a new Feature or alias.
- **Information Responsibility**: Mobile Service Capacity, Staff Operational Presence Register, Zone Allocation Matrix, Mobile Visit Progression, Exchanged Mobile Work-Management Information. Originating work/assignment authority remains with its external/business work-management source.

2.10 On-Call Management

- **Owning Capability**: `On-CallManagement`
- **Functions & Exposed Services**:
  - **Feature: Active On-Call Provider Resolution**:
    - *Function*: `ResolveActiveOn-CallCoverage` — Resolves primary, secondary, and tertiary on-call clinical specialists for emergency escalation.
    - *Exposed Service*: `On-CallSpecialistLookup` — Discloses active on-call contacts for switchboard and emergency teams.
- **Information Responsibility**: On-Call Roster Projection, Escalation Contact Matrix.

2.11 Work Allocation & Dispatch

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Key Architectural Semantics**:
  - **Ownership Guardrail**: Owns the dispatching, queuing, assignment, and operational progression of non-clinical work orders (e.g., patient transfers, cleaning, specimen pickups). It does **not** acquire ownership of Practitioners, Locations, Services, or Credentials.
  - **Capacity Consumer**: May use context-specific capacity/availability in operational allocation; it neither establishes that capacity nor acquires clinical-work allocation authority through mobile integration.
- **Functions & Exposed Services**:
  - **Feature: Work Item Instantiation**:
    - *Function*: `DispatchOperationalWorkOrder` — Allocates work requests to available wardspersons or support teams using applicable allocation policies, staff roles, credentials, location and skill, and delivers allocated work to the target worker, device or team queue.
    - *Exposed Service*: `WorkDispatchService` — Delivers allocated work and captures distinct technical and operational acknowledgements; acknowledgement alone does not establish completion.
    - *Additional Feature relationships*: `FEAT-HSO-20 – Worker Matching & Allocation` is realised by the existing allocation behaviour; `FEAT-HSO-21 – Work Dispatch Delivery` by its dispatch exposure and acknowledgement capture. Applicable credential/privilege determinations remain separately governed; Role information is not itself privilege.
  - **Feature: Work Progress Oversight & Escalation**:
    - *Function*: `ManageOperationalWorkProgression` — Tracks execution states: *Queued → Assigned → Accepted → In-Progress → Completed / Aborted*.
    - *Exposed Service*: `OperationalWorkStatusQuery` — Discloses task progression to requesting wards.
    - *Governed Process*: **Operational Work Progression Process**.
- **Information Responsibility**: Operational Work Dispatch Ledger, Job Progression Milestone Ledger.

2.12 Credential Management

- **Owning Capability**: `CredentialManagement`
- **Functions & Exposed Services**:
  - **Capability-scoped behaviour: Operational Credential Check** (Feature association not established):
    - *Distinction*: Operational badge/access `CredentialManagement` is distinct from Strategy `ClinicalQualificationManagement`. Lexical similarity does not bind this behaviour to qualification/competency Feature `FEAT-HSO-23`.
    - *Function*: `VerifyOperationalAccessCredential` — Verifies physical access badges, specialty ward entry authorizations, and restricted area permissions.
    - *Exposed Service*: `OperationalCredentialCheckpointService` — Evaluates physical/operational access rights.
- **Information Responsibility**: Operational Access Authorization Ledger, Badge Credential Binding Register.

2.13 Patient Transport

- **Owning Capability**: `PatientTransport`
- **Functions & Exposed Services**:
  - **Feature: Transport Dispatch & Progress Tracking**:

    - *Function*: `ManagePatientTransportProgression` — Coordinates intra-facility and inter-facility patient movements: *Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Handover Completed*.
    - *Exposed Service*: `PatientTransportTrackingService` — Provides real-time transit telemetry.
    - *Governed Process*: **Patient Transport Process**.
    - *Additional Feature responsibility*: `FEAT-HSO-24 — Transport Request Ingestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.
- **Information Responsibility**: Patient Transport Request Master, Transit Milestone Log.

2.14 Clinical Logistics Coordination

- **Owning Capability**: `ClinicalLogisticsCoordination`
- **Functions & Exposed Services**:
  - **Feature: Pathology Specimen Transport Tracking**:
    - *Function*: `ManageSpecimenTransportProgression` — Tracks the physical transit of pathology bio-specimens, blood products, and surgical trays: *Collected → Courier Picked Up → In-Transit → Laboratory Ingress Received*.
    - *Exposed Service*: `SpecimenTransitTrackingService` — Exposes chain-of-custody tracking.
    - *Governed Process*: **Specimen Transport Process**.
- **Information Responsibility**: Specimen Courier Manifest, Chain-of-Custody Tracking Register.

2.15 Discharge Management

- **Owning Capability**: `DischargeManagement`
- **Capacity/Progression Consumer**: May use applicable service capacity and patient progression in discharge coordination. It does not establish the consumed capacity or originating clinical clearances; clinical progression, operational readiness, discharge authorisation and physical departure remain distinct.
- **Functions & Exposed Services**:
  - **Feature: Discharge Readiness & Coordination Oversight**:
    - *Function*: `ManageDischargeCoordinationProgression` — Synchronises multi-agency discharge readiness (**illustrative preparation; publication, authorisation and confirmed exit are distinct**): *Discharge Planning Initiated → Medications Reconciled → Transport Booked → Discharge Summary Finalised → Physically Departed*.
    - *Additional Feature relationship*: `FEAT-HSO-27 — Discharge Milestone Tracking` is realised by this existing Function/Process's clinical readiness, medications, transport/services and home-care confirmation milestones.
    - *Departure-triggered responsibility*: `FEAT-HSO-29 — Discharge Notification Dispatch` requires this behaviour to dispatch departure notifications, updated bed states and finalised discharge summaries to external care providers upon confirmed physical exit. Preparation/publication of applicable information may occur earlier; it does not satisfy this departure-triggered responsibility.
    - *Scope qualification*: Preparation, summary finalisation/signing, availability/publication, applicable discharge authorisation and confirmed physical departure remain distinct. The displayed preparation pathway is illustrative, not a universal state machine. Discharge readiness or available information does not establish discharge authorisation or departure.
    - *Exposed Service*: `DischargeReadinessTelemetry` — Discloses discharge barrier checklists and planned departure times.
    - *Governed Process*: **Discharge Progression Process**.

- **Information Responsibility**: Discharge Readiness Checklist Ledger, Estimated Date of Discharge (EDD) Register.

2.16 Clinical Qualification Management

- **Owning Capability**: `ClinicalQualificationManagement`.
- **Functions**:
  - **Feature: Operational Credential Check**:
    - *Function*: `VerifyClinicalQualificationforWorkDispatch` — Validates that a practitioner possesses active clinical credentials and required competency certificates before dispatch of specialised work.
    - *Derivation*: `FEAT-HSO-23` belongs to Strategy Clinical Qualification Management. Provider Administration establishes professional qualification/registration and privilege information, but the dispatch-specific qualification/competency checkpoint was not expressed by that information management alone.
    - *Boundary*: This checkpoint does not originate qualifications, confer clinical privileges, manage clinical competency training or establish a provider Service dependency. Clinical Privilege and Operational Privilege remain distinct, and an activity may require both. Physical badge/access Credential Management in §2.12 remains independently owned and is not associated with this Feature.
- **Information Responsibility**: Qualification/Competency Checkpoint Outcome and Applicable Work Context; source qualification and privilege information retains its established authority and ownership.

The section number is navigation only. No Capability Tier, ancestry, structural ID, new Role, Service or Process is allocated by this directly derived Function.

49. Intrinsic / Shared Enablement — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/05-intrinsic-enablement.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Intrinsic / Shared Enablement — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Intrinsic and Shared Enablement encompasses the horizontal platform capabilities that provide foundational security, longitudinal clinical record assembly, information exchange, workflow orchestration, collaborative spaces, and semantic governance across all healthcare contexts.

The Cross-Cutting Boundary Guardrail

> **These capabilities are bounded architectural responsibilities whose services are widely consumed across the HIE. Delivering a shared service (such as security enforcement, message exchange, workflow coordination, or UI presentation) does not grant ownership of the clinical payloads, business activities, or decisions being processed.**

2. Intrinsic Enablement Capabilities

RAD source listing (presentation only; no inferred ancestry).

Intrinsic / Shared Enablement Capabilities

1. Patient Clinical Record
2. Health Information Exchange (HIE)
3. Health Information Access
4. Health Information Communication
5. Health Information Control
6. Clinical Knowledge Services
7. Clinical Collaboration
8. General Collaboration
9. Workflow & Activity Coordination
10. Calendar Management
11. Presentation Services
12. Information Design Governance

2.1 Patient Clinical Record

- **Owning Capability**: `PatientClinicalRecord`
- **Architectural Mandate**: Governs the canonical, longitudinal, cross-institutional health record for every patient across the healthcare region.
- **Functions & Exposed Services**:
  - **Feature: Longitudinal Health Record (LHR) Assembly**:
    - *Function*: `AssembleLongitudinalClinicalRecord` — Synthesises clinical documents, encounters, diagnostic reports, and medication histories across disparate source systems into a unified chronological clinical view.
    - *Exposed Service*: `LongitudinalClinicalRecordQuery` — Discloses the comprehensive clinical timeline to authorised clinicians.
  - **Feature: Active Clinical Record Access**:
    - *Function*: `MaintainActiveClinicalRecord` — Maintains active problem lists, current medication regimens, allergies, and open clinical alerts for active care coordination.
    - *Exposed Service*: `ActiveProblem&AllergySummaryQuery` — Exposes immediate active clinical summaries.
  - **Feature: Durable Clinical Record Preservation**:
    - *Function*: `PreserveDurableClinicalRecord` — Preserves the governed longitudinal representation and durable, immutable, append-only history of what Harmonia received, knew, asserted or managed, with provenance and versioning. Corrections/supersession establish changed knowledge without retrospectively altering that history or acquiring originating clinical authority.
    - *Exposed Service*: `HistoricalClinicalRecordAccess` — Provides governed historical record retrieval for authorised clinical and legal review; preservation does not itself establish legal qualification or originating authority.
- **Information Responsibility**: Governed Longitudinal Health Record (LHR), Canonical Clinical Timeline, Active Problem List, Immutable Historical Record Store.

2.2 Health Information Exchange (HIE)

- **Owning Capability**: `HealthInformationExchange`
- **Architectural Mandate**: Manages the ingestion, transformation, routing, addressed distribution, and publish-subscribe syndication of health information across organizational and jurisdictional boundaries.
- **Ownership Invariant**: Health Information Exchange owns **exchange transaction state, delivery receipts, and routing policies**. It does **not** acquire ownership of the business information transported.
- **Functions & Exposed Services**:
  - **Feature: Direct Submission Ingestion & Delivery**:
    - *Function*: `ReceiveInformationSubmission` — Validates and ingests standards-compliant clinical messages from external health providers.
    - *Exposed Service*: `HIESubmissionGatewayService` — Ingress endpoint for regional data contributions.
  - **Feature: Clinical Record Retrieval Mediation**:
    - *Function*: `MediateInformationRetrieval` — Federates queries across participating hospital and diagnostic node repositories.
    - *Exposed Service*: `FederatedHealthInformationQuery` — Resolves distributed record queries.
  - **Feature: Addressed Clinical Distribution**:
    - *Function*: `DistributeAddressedInformation` — Directs point-to-point clinical documents to specific recipient practitioner endpoints.
    - *Exposed Service*: `AddressedDocumentDeliveryService` — Securely delivers documents to designated recipient mailboxes.
  - **Feature: Event-Driven Syndication**:

    - *Function*: `SyndicateInformationChange` — Broadcasts event notifications and record updates to subscribed regional care network systems.
    - *Exposed Service*: `ClinicalEventSyndicationStream` — Emits publish-subscribe clinical event feeds.
- **Information Responsibility**: Exchange Transaction Ledger, Route Resolution Map, Subscription & Syndication Matrix.

2.3 Health Information Access

- **Owning Capability**: `HealthInformationAccess`
- **Architectural Mandate**: Provides high-performance search, retrieval, and access-qualified filtering of clinical information across the longitudinal record.
- **Ownership Invariant**: Consumes access control and consent determinations from *Health Information Control*. Search behaviour does **not** own security policy.
- **Functions & Exposed Services**:
  - **Feature: Federated Clinical Query**:
    - *Function*: `SearchHealthInformation` — Executes structured semantic searches across diagnostic reports, clinical notes, and medication histories.
    - *Exposed Service*: `ClinicalInformationSearchService` — Discloses search capabilities to clinical user interfaces.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-ISE-09 — Access-Controlled Information Filtering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.
    - *Function*: `RetrieveHealthInformation` — Fetches discrete clinical observations and original attachments.
    - *Exposed Service*: `ClinicalResourceRetrievalService` — Delivers structured clinical information to requesting applications.
  - **Capability-scoped behaviour: Access-Qualified Result Filtering** (Feature identity not established):
    - *Function*: `ApplyAccess-QualifiedResultFiltering` — Redacts or masks search results and clinical content based on the caller's verified security context and patient consent directives.
    - *Exposed Service*: *(Embedded within Search and Retrieval Services)*.
- **Information Responsibility**: Query Execution Context, Filtered Result Projection State.

2.4 Health Information Communication

- **Owning Capability**: `HealthInformationCommunication`
- **Architectural Mandate**: Encapsulates standards-based health information communication to mediate technical transmission between participating parties.
- **Ownership Invariant**: Does **not** own every business interaction communicated through it; provides protocol adaptation and secure transport mediation.
- **Functions & Exposed Services**:
  - **Feature: Standards-Based Boundary Exchange**:
    - *Function*: `MediateStandards-BasedHealthInformationCommunication` — Handles standards-based communication mediation, transaction coordination, and technical error handling.
    - *Exposed Service*: `StandardsCommunicationGatewayService` — Protocol interface for external client integrations.
- **Information Responsibility**: Transport Protocol Binding Map, Network Session State.

2.5 Health Information Control

- **Owning Capability**: `HealthInformationControl`
- **Architectural Mandate**: Enforces enterprise default-deny authorization, ABAC/RBAC policy evaluation, consent restriction enforcement, immutable security context propagation, and non-repudiation audit logging.
- **Functions & Exposed Services**:
  - **Feature: Health Information Access & Consent Control**:
    - *Function*: `EvaluateInformationAccessAuthority` — Evaluates user role, organisation, patient relationship, purpose of use, and break-glass overrides against default-deny policies.
    - *Exposed Service*: `PolicyEvaluation&AuthorisationService` — Evaluates access requests across the platform.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-ISE-12 – Security Context Propagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.
    - *Function*: `EvaluateConsentConstraint` — Cross-references access requests against client consent directives and sensitive health category restrictions.
    - *Exposed Service*: `ConsentEnforcementService` — Returns masking and redaction instructions.
  - **Capability-scoped behaviour: Security Context Binding** (Feature identity not established):
    - *Function*: `PropagateSecurityContext` — Generates and binds canonical, immutable security contexts to all intra-platform transactions.
    - *Exposed Service*: `SecurityContextValidationService` — Verifies transaction security credentials.
    - *Exposure qualification*: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.
  - **Feature: Non-PHI Compliance Auditing**:
    - *Function*: `RecordSecurity-SignificantActivity` — Records tamper-evident audit evidence (who, what, when, why, patient MRN, policy outcome) without logging unmasked PHI.
    - *Exposed Service*: `SecurityAuditIngress` — Ingests audit events across all capabilities.
- **Information Responsibility**: Authorisation Policy Ruleset, Security Context Tokens, Tamper-Evident Security Audit Trail.

2.6 Clinical Knowledge Services

- **Owning Capability**: `ClinicalKnowledgeServices`
- **Architectural Mandate**: Resolves canonical medical ontologies (SNOMED-CT, LOINC, ICD-10, AMT) and executes cross-terminology concept mappings.
- **Functions & Exposed Services**:
  - **Feature: Clinical Concept Resolution**:
    - *Function*: `ResolveClinicalConcept` — Validates concept identifiers and retrieves preferred clinical rubrics and hierarchy paths.
    - *Exposed Service*: `TerminologyConceptResolutionService` — Exposes concept validation lookups.
  - **Feature: Bidirectional Terminology Mapping**:
    - *Function*: `MapClinicalConcept` — Translates local lab codes and legacy terms to canonical SNOMED/LOINC codes using governed translation maps.
    - *Exposed Service*: `TerminologyMappingService` — Translates codes across terminology systems.

- **Information Responsibility**: Canonical Clinical Ontologies, Semantic Value Sets, Cross-Terminology Mapping Tables.

2.7 Clinical Collaboration

- **Owning Capability**: `ClinicalCollaboration`
- **Architectural Mandate**: Governs real-time multidisciplinary clinical collaboration channels, patient care-team virtual workspaces, and secure clinical discussions.
- **Key Architectural Semantics**:
  - **Discourse vs. Record Guardrail**: Real-time collaborative discussion and messaging in team channels do **not** automatically constitute authoritative clinical health records or observations. Authoritative entries require explicit formal document submission.
- **Functions & Exposed Services**:
  - **Feature: Care-Team Clinical Collaboration Coordination**:
    - *Function*: `CoordinateCare-TeamCollaboration` — Manages care-team communication spaces, participant memberships, and clinical handovers.
    - *Exposed Service*: `ClinicalCollaborationSpaceService` — Manages team collaboration spaces and discussions.
  - **Feature: In-Conversation LHR Query Resolution**:
    - *Function*: `ResolveLHRQuerywithinCollaboration` — Contextually projects active patient summaries directly into collaborative clinical discussion channels.
    - *Exposed Service*: `CollaborationClinicalSummaryResolution` — Embeds clinical summaries in collaboration feeds.
    - *Established ownership*: `Clinical Collaboration → In-Conversation LHR Query Resoluti on [FEAT-ISE-17] → Resolve LHR Query within Collaboration → Collaboration Clinical Summary Resolution`.
    - *Unresolved dependency*: The precise source Service/provider, direct versus mediated retrieval, formal consuming Capability/Function for the exposed Service and intended referent of the withdrawn dependency row remain unestablished. Information ownership, Function ownership, Service ownership, consumption, information use and presentation are distinct; no substitute dependency is implied.
  - **Feature: Zero-PHI Collaboration Metadata Governance**:
    - *Function*: `GovernCollaborationMetadata` — Enforces privacy policies, participant access gates, and zero-PHI space metadata rules.
    - *Exposed Service*: `CollaborationSpaceGovernanceService` — Governs space lifecycles and membership.
- **Information Responsibility**: Collaboration Space Metadata, Care-Team Channel Ledger, Discussion Thread Index.

2.8 General Collaboration

- **Owning Capability**: `GeneralCollaboration`
- **Functions & Exposed Services**:
  - **Feature: Operational Team Chat Enablement**:
    - *Function*: `CoordinateOperationalCollaboration` — Provides communication spaces for facility logistics, disaster management teams, and administrative committees.
    - *Exposed Service*: `OperationalCollaborationService` — Provides non-clinical operational messaging channels.
- **Information Responsibility**: Operational Channel Index, Non-Clinical Messaging Log.

2.9 Workflow & Activity Coordination

- **Owning Capability**: `Workflow&ActivityCoordination`

- **Architectural Mandate**: Coordinates execution state, task dispatch, timers, deadlines, and escalations across three distinct activity archetypes:
  1. **Work Order**: Assigned physical/human operational tasks (*human doing* — e.g., transport a patient, clean a bay).
  2. **To Do**: Assigned clinical review or administrative decision tasks (*human reviewing/updating/deciding* — e.g., review abnormal lab result, countersign discharge summary).
  3. **Synthetic Task**: Automated system-executable activities (*non-human executable work* — e.g., generate summary, syndicate batch, evaluate rules).
- **Ownership Invariant**: Workflow & Activity Coordination owns **generic activity coordination semantics and timers**. It does **not** acquire ownership of the business meaning or clinical outcome of the activity.
- **Clinical-work boundary**: Coordination supports externally established work requirements and authorised human review/update/approval. It does not determine clinical tasks, manage clinical handover or own EMR clinical worklists. The clinical-work integration rule preserves clinical authority and the three work-unit meanings; Task remains synthetic work.
- **Outcome uncertainty**: For materially significant managed activity outcomes, missing acknowledgement, response or observation does not establish success or failure. Preserve uncertainty pending authoritative observation/reconciliation. This is a Business obligation, not a universal new lifecycle state; assurance evidence insufficiency remains distinct from execution uncertainty.
- **Functions & Exposed Services**:
  - **Feature: Work Order Progression**:
    - *Function*: `CoordinateWorkOrder` — Coordinates operational task lifecycles and dispatches.
    - *Exposed Service*: `WorkOrderCoordinationService` — Manages operational work progression.
    - *Governed Process*: **Work Order Progression Process**.
  - **Feature: To Do Decision & Approval Coordination**:
    - *Function*: `CoordinateToDo` — Coordinates human review/update/approval items, reminders and sign-off queues within Harmonia's operational responsibility; clinical requirements and decisions remain with accountable clinical authority.
    - *Exposed Service*: `ToDoManagementService` — Provides To Do inbox and sign-off coordination without acquiring underlying clinical-work management.
    - *Governed Process*: **To Do Progression Process**.
  - **Feature: Synthetic Task Orchestration**:
    - *Function*: `CoordinateSyntheticTask` — Orchestrates automated system workflows and operational progression.
    - *Exposed Service*: `SyntheticTaskExecutionService` — Coordinates automated tasks.
    - *Governed Process*: **Synthetic Task Progression Process**.
  - **Feature: Workflow Timeout & Escalation Oversight**:
    - *Function*: `SuperviseActivityTimeout/Escalation` — Monitors SLAs and triggers escalations when clinical or operational deadlines elapse.
    - *Exposed Service*: `ActivityEscalationTelemetry` — Discloses SLA breaches.
- **Information Responsibility**: Generic Activity Instance Registry, Task State Progression Graph, SLA Deadline & Timer Ledger.

2.10 Calendar Management

- **Owning Capability**: `CalendarManagement`
- **Architectural Mandate**: Projects unified chronological timelines across appointments, on-call rosters, and scheduled operational events.
- **Ownership Invariant**: Does **not** acquire authoritative ownership of rosters or appointment bookings (which remain owned by source scheduling and workforce systems).
- **Functions & Exposed Services**:
  - **Feature: Schedule Projection Integration**:

  - *Function*: `ProjectOperationalSchedule` — Aggregates and overlays multidisciplinary schedules onto a unified temporal view.
  - *Exposed Service*: `UnifiedCalendarProjectionQuery` — Discloses consolidated temporal schedules.
- **Feature: Temporal Event Correlation**:
  - *Function*: `CorrelateActivitywithTemporalContext` — Associates clinical events with shift boundaries and appointment slots.
  - *Exposed Service*: `TemporalContextResolution` — Resolves operational shifts.
- **Information Responsibility**: Projected Unified Timeline, Temporal Correlation Index.

2.11 Presentation Services

- **Owning Capability**: `PresentationServices`
- **Architectural Mandate**: Delivers user-facing portals, clinical dashboards, administrative consoles, and mobile views.
- **Ownership Invariant**: Presentation owns **rendering, user interaction, and layout concerns**. It does **not** own the underlying clinical or operational information being rendered.
- **Functions & Exposed Services**:
  - **Feature: Longitudinal Clinical & Operational Presentation**:
    - *Function*: `PresentContextualHealthandOperationalInformation` — Composes clinical summaries, work queues, and patient records into responsive role-based user interfaces.
    - *Exposed Service*: `ContextualPresentationService` — Delivers contextual health and operational information for presentation.
- **Information Responsibility**: UI View Configuration, User Preference Profile, Presentation Layout Specification.

2.12 Information Design Governance

- **Owning Capability**: `InformationDesignGovernance`
- **Architectural Mandate**: Governs the enterprise canonical information model, data definitions, constraint schemas, terminology bindings, and exchange profiles.
- **Functions & Exposed Services**:
  - **Feature: Information Standards & Semantic Governance**:
    - *Function*: `GovernInformationSemantics` — Authors and validates canonical data concepts and entity relationships.
    - *Exposed Service*: `SemanticModelDefinitionService` — Discloses canonical model definitions.
  - **Capability-scoped behaviour: Information Definition & Schema Governance** (Feature identity not established):
    - *Function*: `GovernInformationDefinition` — Maintains governed data element dictionaries, data types, and structural rules.
    - *Exposed Service*: `InformationDictionaryQuery` — Provides data dictionary specifications.
  - **Capability-scoped behaviour: Terminology Binding Governance** (Feature identity not established):
    - *Function*: `GovernTerminologyBinding` — Binds canonical data elements to authoritative terminology value sets.
    - *Exposed Service*: `TerminologyBindingSpecificationLookup` — Discloses value set bindings.
  - **Capability-scoped behaviour: Exchange Profile Governance** (Feature identity not established):
    - *Function*: `GovernExchangeProfile` — Defines standards-based interchange profiles and message constraints.

    - *Exposed Service*: `ExchangeProfileSpecificationQuery` — Discloses interoperability schemas.
  - **Feature: Semantic Conformance Verification**:
    - *Function*: `VerifySemanticConformance` — Validates candidate messages against canonical governance schemas.
    - *Exposed Service*: `SemanticConformanceVerificationService` — Validates representations for compliance.
- **Information Responsibility**: Enterprise Canonical Semantic Model, Governed Data Dictionary, Terminology Binding Matrix, Exchange Profile Catalogue, Semantic Conformance Ruleset.

50. Canonical Enterprise Capability Model (EC-01 .. EC-14)

RAD source: `docs/markdown/02-strategy/capabilities/enterprise-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Enterprise Capability Model (EC-01 .. EC-14)

Overview & Purpose

The Enterprise Capability Tier represents **reusable ICT architectural functionality** that recurs across multiple distinct healthcare business enabling capabilities and features.

Enterprise Capabilities answer the fundamental architectural question:

> **"How is this function delivered across our systems?"**

Where Business Enabling capabilities are defined within authentic clinical, operational, and administrative healthcare contexts (e.g., Ward Census, Medication Administration, Closed-Loop Orders), Enterprise Capabilities represent the **horizontal, domain-neutral capabilities** through which those healthcare features are delivered.

RAD source listing (presentation only; no inferred ancestry).

> BUSINESS ENABLING FEATURES (Hundreds of domain-specific behaviours)
>
> Example: Patient Bed Turnover, Pathology Order, Practitioner Lookup
>
> derived across features by asking:
>
> "How is this function delivered?"
>
> ▼
>
> ENTERPRISE CAPABILITY TIER (EC-01 .. EC-14)
>
> 14 Reusable, Technology-Neutral Architectural Capabilities
>
> EC-01 .. EC-13 inform clustering of
>
> ▼
>
> STRATEGIC LOGICAL RESPONSIBILITY MODEL
>
> Mneme, Mnemosyne, Ponos, Pylai, Calliope, Iris, Digital Twin

The component-clustering progression describes established EC-01 through EC-13 derivations only. EC-14 is added by the approved Health Service Assurance Strategy derivation; no strategic logical or application component is allocated EC-14 by this diagram or catalogue.

Derivation Methodology & Architectural Principles

The established EC-01 through EC-13 were derived by systematically analyzing the system-enabled features across all five Business Enabling views. EC-14 Service Guardian is the approved addition derived from REQ-FND-005, BC-18 and the three assurance Business Enabling Capabilities. Their first-pass contribution model records the explicitly approved relationships without inventing intermediate Features or component allocations. The derivation adheres to five strict architectural principles:

1. Vertical Recurrence Test

A functional requirement qualifies as an Enterprise Capability only if it recurs vertically across multiple, diverse healthcare domains.

- If a function appears only within laboratory orders, it remains domain-specific Business Enabling behaviour.
- If a function recurs across clinical document versioning, directory update proposals, and bed state turnover, it reflects an underlying Enterprise Capability (**EC-03 Managed State & Lifecycle**).

2. Behavioural Properties vs. Top-Level Capabilities

Not every recurring function or algorithmic mechanism warrants promotion to a standalone top-level Enterprise Capability:

- **Correlation** is an intrinsic behavioural property appearing across multiple capabilities (e.g., entity correlation in EC-01, event correlation in EC-09, context correlation in EC-02), **not** a standalone `CorrelationService`.
- **Acknowledgement** is a reusable interaction pattern within exchange semantics (EC-08) and operational activity tracking (EC-10), **not** an independent capability.
- **Assignment and Dispatch** are core execution functions within **EC-10 Activity & Execution**.
- **Indexing and Caching** are technical enabling mechanisms within **EC-04 Information Management** and **EC-05 Search & Discovery**, not top-level architectural capabilities.
- **Retry and Circuit Breaking** are resilience properties within **EC-12 Operational Assurance**.

3. Reusable Capability ≠ Centralised Service

> **An Enterprise Capability describes reusable architectural functionality; it does NOT imply a monolithic, centralized runtime service.**

Enterprise Capabilities represent bounded reusable responsibilities. Their realisation may require participation, consumption or enforcement across multiple architectural elements, but this does not imply shared semantic ownership or prohibit establishment of a responsibility centre.

Distributed participation does not imply distributed responsibility. Cross-cutting concern does not imply cross-cutting responsibility.

Examples of participation in established reusable responsibilities include:

- **EC-02 Context Management**: Context is established at ingress (Pylai), maintained during active access (Mneme), propagated across execution units (Ponos), and preserved in audit (Kleio) where required. These participation points do not require a monolithic Context Service or establish shared semantic ownership.
- **EC-06 Policy & Control**: Policy evaluation guards ingress boundaries, persistence interfaces and presentation gateways while retaining bounded policy responsibility.
- **EC-07 Provenance & Traceability**: Required attribution and meaningful evidence are preserved at significant boundaries and transformations; routine technical activity does not automatically require durable evidence.
- **EC-12 Operational Assurance**: Resilience, duplicate suppression and concurrency integrity may require enforcement across messaging, storage and execution elements without distributing semantic ownership.

Responsibility centres follow established architectural responsibility. No capability is allocated merely to complete a diagram, and distributed participation does not prohibit a responsibility centre.

4. Zero Technology Leakage

Enterprise Capabilities are technology-neutral:

- They are free of concurrency primitives (threads, worker thread pools, execution daemons).
- They are free of message broker constructs (topics, queues, partitions, JMS headers).
- They are free of storage technologies (PostgreSQL, Infinispan, JPA, DDL, table locks).
- They are free of specific software packages (HAPI FHIR, Apache Camel, Netty, Spring Boot, Vue 3).

5. Capabilities vs. Software Components

An Enterprise Capability describes *what* functionality is provided. Subsystems (such as Pylai, Ponos, Mneme, Mnemosyne, Calliope, and Iris) represent *strategic logical component responsibility centres* defined in the existing Strategic Logical Component Responsibility Model. Software components are consumers, aggregators, or providers of capabilities; **a software component name is never a capability definition**.

The 14 Canonical Enterprise Capabilities

RAD source listing (presentation only; no inferred ancestry).

EC-01 Managed Entity & Relationship

EC-02 Context Management

EC-03 Managed State & Lifecycle

EC-04 Information Management

EC-05 Search & Discovery

EC-06 Policy & Control

EC-07 Provenance & Traceability

EC-08 Interoperability & Exchange

EC-09 Event & Subscription

EC-10 Activity & Execution

EC-11 Interaction & Experience

EC-12 Operational Assurance

EC-13 Semantic Governance & Conformance

EC-14 Service Guardian

EC-01: Managed Entity & Relationship

- **Architectural Scope**: Identify, classify, relate, and maintain the structural integrity of managed healthcare and administrative entities throughout the enterprise landscape.
- **Functional Responsibilities**:
  - Uniquely identifying core domain entities (Persons, Practitioners, Roles, Organisations, Locations, Services, Devices).
  - Resolving and maintaining multi-authority identifier systems and aliases without destructive overwriting.
  - Representing and traversing multi-dimensional relationships (affiliations, organizational hierarchies, physical location trees, care team memberships).
  - Maintaining referential integrity across interconnected entity graphs.
- **Anti-Responsibilities**: Does not own physical database relational tables or foreign key constraints (Domain 04/07).

EC-02: Context Management

- **Architectural Scope**: Establish, validate, propagate, and preserve the execution, clinical, security, and administrative context within which information and activities have meaning.
- **Functional Responsibilities**:
  - Binding the subject of care, encounter, ordering provider, and healthcare service context to in-flight messages and transactions.
  - Propagating tamper-evident, attributable security context across asynchronous, multi-hop operational boundaries.
  - Ensuring context preservation during cross-protocol transformations across disparate healthcare exchange formats and protocols.
  - Detecting and rejecting requests where mandatory operational or clinical context is missing or inconsistent.
- **Collaborative Nature**: Bounded reusable context responsibility may be realised through participation across gateways, activity execution and information-access boundaries. Participation does not transfer semantic ownership.

EC-03: Managed State & Lifecycle

- **Architectural Scope**: Govern the current and historical state, effective temporal periods, state transition validations, and lifecycle progression of managed integration artifacts and entities.
- **Functional Responsibilities**:
  - Defining and enforcing valid state transition state machines (e.g., `REQUESTED` → `VALIDATED` → `IN_PROGRESS` → `COMPLETED`).
  - Maintaining temporal validity intervals (effective start and end dates) for relationships, credentials, and states.
  - Supporting non-destructive state progression, preserving historical state versions for longitudinal review.
  - Rejecting illegal or out-of-order state transitions while accommodating idempotent replay.
- **Anti-Responsibilities**: Does not define thread-level mutexes or database transaction isolation levels.

EC-04: Information Management

- **Architectural Scope**: Validate, represent, persist, retrieve, version, and manage governed clinical, administrative, and operational information assets.
- **Functional Responsibilities**:
  - Validating inbound information structures against canonical information constraints.
  - Providing authoritative durable persistence for historical health records and administrative registries.
  - Managing active distributed working memory representations for low-latency retrieval.
  - Governing document versioning, supersession, amendments, and retractions without data loss.
- **Architectural Principle**: Enforces the foundational separation between *active distributed state access* and *authoritative durable state preservation* (Axiom AX-05).

EC-05: Search & Discovery

- **Architectural Scope**: Index, search, resolve, filter, and discover information assets, entities, directory endpoints, and healthcare services across the enterprise.
- **Functional Responsibilities**:
  - Providing multi-criteria indexed search across complex entity graphs (e.g., find active cardiologists at Location X accepting new referrals).

  - Resolving service and practitioner communication endpoints for addressed delivery.
  - Supporting bounded, paginated clinical queries across longitudinal patient records.
  - Applying dynamic security and consent filters to search results prior to egress.
- **Anti-Responsibilities**: Does not dictate specific search engine indexes, inverted index implementations, or SQL query syntaxes.

EC-06: Policy & Control

- **Architectural Scope**: Evaluate and enforce security, consent, privacy, and regulatory policies including authority, access control, and information handling rules.
- **Functional Responsibilities**:
  - Executing impartial, default-deny policy evaluations for all integration transactions.
  - Enforcing Role-Based and Attribute-Based Access Control (RBAC/ABAC) policies.
  - Evaluating patient consent directives and privacy restrictions at boundary and query checkpoints.
  - Enforcing statutory information disclosure boundaries (e.g., mental health, VIP records).
- **Collaborative Nature**: Evaluated across boundary gateways, internal bus dispatch, storage interfaces, and presentation layers.

EC-07: Provenance & Traceability

- **Architectural Scope**: Preserve required origin attribution, meaningful transformation history, significant execution checkpoints, tamper-evident audit evidence and end-to-end traceability. Harmonia SHALL preserve provenance, audit and operational evidence where required by governing obligations, architectural significance, accountability, reconstruction, security or assurance needs. Routine technical activity SHALL NOT automatically require durable evidentiary recording.
- **Functional Responsibilities**:
  - Recording attributable, verifiable origin metadata for information sources, authors, and timestamps.
  - Tracking information-significant transformation history and required mapping checkpoints as information crosses system seams.
  - Capturing non-PHI operational and security audit trails for regulatory compliance (HIPAA, GDPR).
  - Supporting end-to-end transaction tracing across asynchronous processing pipelines.
- **Collaborative Nature**: Participating elements preserve required provenance and audit context according to policy. Telemetry, operational facts, provenance, audit and assurance evidence remain distinct. Being evidence is contextual, not an intrinsic property of every recorded fact; preservation does not establish an assurance conclusion.

EC-08: Interoperability & Exchange

- **Architectural Scope**: Resolve destinations, transform schemas, route, transport, deliver, acknowledge, and coordinate standards-based information exchange across enterprise boundaries.
- **Functional Responsibilities**:
  - Providing multi-protocol boundary adaptation for healthcare standards (HL7 v2, FHIR REST, DICOM, secure messaging).
  - Executing bidirectional structural and semantic transformations between legacy formats and canonical representations.
  - Ensuring reliable addressed delivery and fan-out distribution to external clinical endpoints.
  - Processing and distinguishing technical delivery acknowledgements from business acknowledgements.

- **Boundary Rule**: Egress terminates Harmonia management of the emitted representation (Axiom AX-13).

EC-09: Event & Subscription

- **Architectural Scope**: Publish, ingest, correlate, filter, and syndicate events and subscription-driven changes across the enterprise landscape.
- **Functional Responsibilities**:
  - Ingesting operational and clinical event triggers from external systems and internal state changes.
  - Managing topic- and criteria-based event subscriptions for internal and external consumers.
  - Evaluating dynamic subscription filters to match event payloads with interested subscriber criteria.
  - Delivering asynchronous event notifications reliably to registered subscribers.
- **Decoupling Note**: Defined strictly in terms of event publishing, criteria matching, and notification delivery; entirely independent of broker topics, partitions, or queues.

EC-10: Activity & Execution

- **Architectural Scope**: Define, instantiate, match, assign, dispatch, supervise, and progress coordinated operational and clinical activity units.
- **Functional Responsibilities**:
  - Instantiating operational work items (Work Orders, To Dos, synthetic platform Tasks) in response to triggers.
  - Matching and assigning work units to eligible human or automated performers and roles based on policy rules.
  - Dispatching work units to their assigned performers with acknowledgement according to the applicable interaction contract.
  - Supervising execution progress, tracking timeouts, managing escalations, and handling activity interruptions.
- **Decoupling Note**: Free of worker threads, thread pools, execution daemons, or specific workflow runtime engines.

EC-11: Interaction & Experience

- **Architectural Scope**: Provide contextual interaction between human actors, Harmonia, and enterprise presentation services, including visualization, alerting, and collaboration integration.
- **Functional Responsibilities**:
  - Mediating presentation interactions via dedicated backend-for-frontend gateways decoupled from internal databases.
  - Rendering clinical timelines, provider directory management interfaces, and operational console dashboards.
  - Providing contextual notifications and alerts to human users across web and mobile channels.
  - Projecting longitudinal record context into secure multidisciplinary collaboration spaces.
- **Boundary Note**: Does not absorb the healthcare semantics of Clinical Collaboration or medical documentation. Consumes established identity and security context; does not own authentication, consent, or clinical truth.

EC-12: Operational Assurance

- **Architectural Scope**: Ensure concurrency integrity, system resilience, duplicate detection, exception handling, automated recovery, and operational telemetry across platform operations.
- **Functional Responsibilities**:
  - Enforcing sliding-window duplicate message detection and idempotent suppression.
  - Guaranteeing ingress dual-write safety (durable acceptance before emitting positive application ACK).
  - Managing structured failure recovery, exception escalation, and dead-letter isolation.
  - Emitting platform telemetry, operational health metrics, and queue processing metrics.
- **Collaborative Nature**: Realized collaboratively across boundary gateways, messaging infrastructure, execution workers, and storage layers.

EC-12 / EC-14 Boundary

EC-12 retains the established scope and responsibilities above. It may contribute directly to generic operational/processing assurance, limited to that contribution in the approved assurance matrix. It does not own the general semantics of assurance disposition, assurance-specific criteria applicability, evidentiary association, assessment against assurance criteria, adjudication, or independent assurance findings and conclusions. Those distinguishing responsibilities are introduced through EC-14 Service Guardian. Operational health and business-level processing assurance remain distinct; EC-12 is not expanded to absorb EC-14.

EC-13: Semantic Governance & Conformance

- **Architectural Scope**: Govern canonical models, schemas, terminologies, value sets, mapping tables, constraint rules, and semantic conformance validation across the enterprise.
- **Functional Responsibilities**:
  - Defining and publishing canonical healthcare data models and profile structures.
  - Governing clinical terminology mappings, value sets, and concept lookups (SNOMED CT, LOINC).
  - Validating inbound and outbound message representations against published semantic rules.
  - Governing semantic versioning, deprecation, and backward compatibility across schema revisions.
- **Component Relationship**: Informs the Calliope strategic logical component, but Calliope is the responsibility centre, not the capability definition.

EC-14: Service Guardian

- **Architectural Scope**: Service Guardian provides the reusable capability to independently establish whether governed service behaviour, state or outcomes remain within defined assurance expectations by associating applicable criteria and trustworthy evidence with a governed subject, assessing that evidence, adjudicating the assessment, and establishing reportable assurance findings and conclusions.
- **Functional Responsibilities**:
  - Supplying assurance-specific semantics for assurance disposition and assurance criteria applicability.
  - Associating a governed subject/context with applicable criteria and sufficient trustworthy evidence for a particular assurance purpose.
  - Assessing evidence against applicable criteria and adjudicating assessed detail to establish reportable findings, exceptions and conclusions.
  - Preserving assurance independence, temporal/version-specific evidence and criteria applicability, and explicit inability to establish a conclusion where evidence is insufficient.

- **Collaborative Nature**: Supplies the assurance-specific semantics required to bind existing reusable capabilities into governed assurance. It contributes directly to Assurance Design, Assurance Criteria Management and Governed Assurance; direct contribution is not sole realisation. The approved matrix preserves the contributions of EC-01 through EC-13.
- **Anti-Responsibilities**: Does not own the underlying governed subject, authoritative governance requirements, or source information used as evidence. Does not perform, manage, remediate, assign, delegate or operationally escalate the subject activity. Does not replace System Operations or EC-12 Operational Assurance, and does not provide Clinical Services Delivery Assurance.
- **Allocation Boundary**: No strategic logical component, application component, runtime service or implementation construct is allocated EC-14. The catalogue defines a capability, not a software component or deployment boundary.

Conceptual Behavioural Sequence

RAD source listing (presentation only; no inferred ancestry).

Define Good Behaviour

↓

Gather Evidence

↓

Assess Evidence against Good Behaviour

↓

Adjudicate Assessed Detail

↓

Report

This sequence explains the capability; it is not an implementation workflow specification or a derived Business Process. “Define Good Behaviour” identifies assurance expectations and criteria; it does not transfer ownership of governing requirements to Service Guardian. Source information ownership remains with its responsible capability; assurance owns its contextual evidentiary association.

The Business Enabling assurance principles govern disposition, generic processing assurance, evidence/criteria temporality, assessment/adjudication, independence, failure/recovery and explicit evidence insufficiency. These semantics are not universally reducible to PASS / FAIL / UNKNOWN and do not imply an assurance information model or persistence mechanism.

Summary Matrix: Enterprise Capabilities

ID	Enterprise Capability Name	Architectural Focus & Essence	Key Recurring Functions
EC-01	Managed Entity & Relationship	Entity identification, graph relationships, referential integrity	Identity resolution, relationship navigation, alias linking
EC-02	Context Management	Establishing, validating, and propagating operational context	Context binding, multi-hop propagation, security tokens
EC-03	Managed State & Lifecycle	State machines, temporal validity, lifecycle progression	State validation, effective periods, non-destructive updates

ID	Enterprise Capability Name	Architectural Focus & Essence	Key Recurring Functions
EC-04	Information Management	Validation, durable persistence, active distributed state	Schema validation, durable preservation and recovery, active access
EC-05	Search & Discovery	Indexing, multi-criteria search, endpoint resolution	Indexed query, criteria filtering, endpoint discovery
EC-06	Policy & Control	Impartial default-deny security, access control, consent	RBAC/ABAC evaluation, consent enforcement, VIP gating
EC-07	Provenance & Traceability	Origin attribution, audit evidence, transformation history	Tamper-evident audit logging, source attribution, chain of custody
EC-08	Interoperability & Exchange	Multi-protocol boundary exchange, transport, delivery	Protocol mediation, transformation, routing, delivery ACK
EC-09	Event & Subscription	Event publishing, criteria filtering, notification dispatch	Subscription management, criteria matching, syndication
EC-10	Activity & Execution	Work unit instantiation, assignment, dispatch, supervision	Matching, dispatching, progress tracking, escalation
EC-11	Interaction & Experience	Decoupled presentation, user interaction, dashboards	BEFE mediation, clinical view rendering, alert display
EC-12	Operational Assurance	Resilience, concurrency integrity, duplicate suppression	Dual-write safety, deduplication, recovery, telemetry
EC-13	Semantic Governance & Conformance	Canonical schemas, terminologies, conformance rules	Schema publishing, concept mapping, constraint checking
EC-14	Service Guardian	Independent governed assurance of service behaviour, state and outcomes	Assurance disposition, criteria applicability, evidentiary association, assessment, adjudication, findings and conclusions

Multi-Capability Composition Dynamics

Enterprise Capabilities do not operate in isolation; they compose harmoniously to deliver complex healthcare features. Below are three representative multi-capability compositions demonstrating this derivation logic:

Composition 1: Person Identifier Resolution

- **Healthcare Need**: Resolving multi-authority identifiers (MRN, national IHI, clinic client IDs) for an incoming patient transaction.
- **Composed Enterprise Capabilities**:
  - `EC-01ManagedEntity&Relationship`: Resolves entity models and alias relationships.
  - `EC-05Search&Discovery`: Searches existing identifier registers and resolves matches.
  - `EC-03ManagedState&Lifecycle`: Validates identifier active/superseded status.
  - `EC-07Provenance&Traceability`: Records origin authority, timestamp, and audit trail.
  - `EC-06Policy&Control`: Evaluates requesting system access permissions.
  - `EC-12OperationalAssurance`: Guarantees deterministic matching under concurrent load.

Composition 2: Closed-Loop Order Progression

- **Healthcare Need**: Ingesting a diagnostic order, routing to a laboratory, and tracking execution through to report outcome.
- **Composed Enterprise Capabilities**:
  - `EC-02ContextManagement`: Binds clinical encounter, patient, and ordering provider context.
  - `EC-05Search&Discovery`: Resolves target laboratory endpoint and service capability.
  - `EC-01ManagedEntity&Relationship`: Links order entity to patient, provider, and service.
  - `EC-08Interoperability&Exchange`: Manages boundary protocol delivery and captures technical ACK.
  - `EC-03ManagedState&Lifecycle`: Enforces order state progression (`PLACED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - `EC-10Activity&Execution`: Instantiates activity units, monitors timeouts, and supervises fulfillment.
  - `EC-07Provenance&Traceability`: Captures end-to-end chain of custody and delivery timestamps.
  - `EC-12OperationalAssurance`: Enforces dual-write safety and duplicate suppression.

Composition 3: Work Allocation & Dispatch

- **Healthcare Need**: Coordinating operational facility tasks (e.g., patient transport, ward bed cleaning).
- **Composed Enterprise Capabilities**:
  - `EC-02ContextManagement`: Binds patient, physical room/bed, and service context.
  - `EC-03ManagedState&Lifecycle`: Progresses work order state (`CREATED` → `DISPATCHED` → `IN_PROGRESS` → `DONE`).
  - `EC-05Search&Discovery`: Discovers available and credentialed staff matching task criteria.
  - `EC-01ManagedEntity&Relationship`: Links assigned worker, patient, location, and equipment.
  - `EC-10Activity&Execution`: Executes allocation logic, worker dispatch, and escalation oversight.
  - `EC-08Interoperability&Exchange`: Delivers work orders to mobile devices or external systems.
  - `EC-12OperationalAssurance`: Prevents duplicate assignment and ensures resilient retry.
  - `EC-07Provenance&Traceability`: Captures worker acceptance and task completion evidence.

Downstream Progression

Enterprise Capabilities represent what reusable functions the platform delivers. The existing Strategic Logical Component Responsibility Model describes the established responsibility compositions, and the Strategic Resources catalogue records resource adjudications. Downstream progression includes:

- **Domains 05–07 (Application, Integration, Technology)**: Implements these capabilities through concrete software packages, integration protocols, and runtime technologies.
- ICT Foundation Lenses: Cross-cutting technical enablement considerations guiding technology realization.
- Capability Tier Progression Model: Detailed vertical derivation rules.

The established component derivations and examples for EC-01 through EC-13 remain unchanged. EC-14 and the three assurance Business Enabling Capabilities stop at the approved Strategy derivation boundary: no allocation to Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or any other logical/application construct is established. No downstream Business, Information or solution architecture is derived here.

51. Domain 03 to Domain 04 Traceability Framework

RAD source: `docs/markdown/04-information-architecture/traceability/domain03-traceability.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Domain 03 to Domain 04 Traceability Framework

1. The Four-Stage Derivation Framework

The Information Architecture (Domain 04) is derived systematically from the business semantics established in the Business Architecture (Domain 03).

Information Concepts, Relationships and Assemblies trace through established business responsibility using the following four-stage derivation framework. It does not allocate Capability Tiers or require invented hierarchy, Feature associations or dependencies:

RAD source listing (presentation only; no inferred ancestry).

1. Business Capability / Feature (Domain 02 / Domain 03)

↓ delivers

2. Business Function / Process (Domain 03)

↓ establishes

3. Business Information Responsibility (Domain 03 Matrix)

↓ realised as

4. Domain 04 Information Concepts, Relationships & Assemblies

RAD diagram: textual source notation.

```
graph TD
    CAP["1. Owning Business Capability / Feature<br/>(Domain 02 / Domain 03)"]
    FUNC["2. Business Function / Stateful Process<br/>(Domain 03 Behaviour & Processes)"]
    RESP["3. Business Information Responsibility<br/>(Domain 03 Ownership Matrix)"]
    D04["4. Domain 04 Information Concepts, Relationships & Assemblies<br/>(Entities, Activities, Events,
    ↪  Assertions, Definitions, Assemblies)"]

    CAP -->|"delivers"| FUNC
    FUNC -->|"establishes & governs"| RESP
    RESP -->|"formally realised as"| D04
```

1.1 Traceability Principles

1. **Derivation, Not Redefinition**: Domain 04 does not invent new business capabilities or alter the ownership boundaries established in Domain 03.
2. **Purity of Information Responsibilities**: The Business Information Responsibility matrix (`docs/markdown/03-business-architecture/information-responsibility/information-responsibility.md`) is the authoritative source for architectural information responsibilities. Architectural information responsibility is a capability property and is distinct from the governance role of `InformationSteward`.
3. **Bounded Responsibility Derivation**: Information Concepts for which Harmonia claims architectural responsibility SHALL trace to an owning Domain 03 Information Responsibility. Referenced, consumed, externally authoritative or contextual Information Concepts may be

represented where required to discharge a traced Harmonia responsibility, but SHALL NOT thereby acquire Harmonia ownership or authority.

4. **Truthful Traceability**: Under AX-17, traceability represents established relationships and explicitly preserves unestablished ones. Unresolved Capability Tier, complete ancestry or full Canonical ID does not prevent derivation from an established owning element and responsibility. Candidates remain non-authoritative until accepted.

2. Representative Derivation Examples

The following representative examples demonstrate how Domain 03 capabilities, functions, and information responsibilities realise concrete Domain 04 Information Concepts, Relationships, and Assemblies:

2.1 Representative Example 1: Health Service Administration (Mandatory Reference)

RAD source listing (presentation only; no inferred ancestry).

Health Service Administration (Capability / Feature: Service-Location-Provider Binding)

↓ delivers

Maintain Service / Location / Provider Map (Function)

↓ establishes

Service / Location / Provider Mapping Matrix (Information Responsibility)

↓ realised as

OfferedHealthcareService (Definition)

DeliverableHealthcareService (Contextual Binding)

Service-Provider-Location Relationships & Qualified Containment

↓ references participating entities; does not own their meaning

Service Provider / Healthcare Organisation (Entity; responsibility retained by its established owner)

Healthcare Location (Entity; Location Administration responsibility retained)

RAD diagram: textual source notation.

```
graph TD
    subgraph D03 ["Domain 03: Business Architecture"]
        HSA["Capability: Health Service Administration"]
        FUNC_MAP["Function: Maintain Service / Location / Provider Map"]
        RESP_MAP["Information Responsibility: Service / Location / Provider Mapping Matrix"]
        HSA --> FUNC_MAP --> RESP_MAP
    end

    subgraph D04 ["Domain 04: Information Architecture"]
        OFFERED["OfferedHealthcareService (Definition)"]
        DELIV["DeliverableHealthcareService (Contextual Binding)"]
        ORG["Healthcare Organisation (Entity)"]
        LOC["Healthcare Location (Entity)"]
        REL_MAP["Information Relationships:<br/>• Organisation.delivers(DeliverableHealthcareService)<br/>•
        ↪  DeliverableHealthcareService.locatedAt(HealthcareLocation)<br/>•
        ↪  OfferedHealthcareService.contains(OfferedHealthcareService)"]
    end

    RESP_MAP -.->|"realised as"| OFFERED
    RESP_MAP -.->|"realised as"| DELIV
    RESP_MAP -.->|"references mapped entity; responsibility retained"| ORG
    RESP_MAP -.->|"references mapped entity; responsibility retained"| LOC
    RESP_MAP -.->|"realised as"| REL_MAP
```

Under the Business ownership matrix, Health Service Administration owns the mapping associations; it does not own the mapped Provider, Location or Practitioner entities. The example references their participation without transferring their established information responsibility or originating authority. This clarification adds no Business relationship or replacement service-binding model.

2.2 Representative Example 2: Person Identity Administration

RAD source listing (presentation only; no inferred ancestry).

Person Identity (Capability / Feature: Cross-Authority Identifier Correlation)

↓ delivers

Correlate Person Identifiers (Function)

↓ establishes

Person Identity Record, Identifier Namespace Bindings, Cross-Authority Correlation Graph (Information Responsibility)

↓ realised as

Person (Entity)

Identifier Namespace Binding (Assertion)

Cross-Authority Correlation Link (Relationship with Qualification & Authority)

2.3 Representative Example 3: Order Administration

RAD source listing (presentation only; no inferred ancestry).

Order Administration (Capability)

↓ established Features

FEAT-SA-11 — Order Request Ingestion

FEAT-SA-12 — Order Destination Resolution & Routing

FEAT-SA-13 — Order Closed-Loop Progression Tracking

FEAT-SA-14 — Order Cancellation & Modification Coordination

↓ established Functions / exposed Services

Receive Order Request / Order Requisition Ingress

Resolve Order Destination / Order Dispatch Service

Manage Order Progression / Order Status & Tracking Query

Associate Order Outcome / Order Outcome Notification (Capability-scoped; Feature association not established)

↓ governed Process

Closed-Loop Order Progression Process

↓ establishes

Clinical Order & Closed-Loop Matrix (Information Responsibility)

Supporting: Clinical Order Master Record, Closed-Loop Tracking Ledger, Order-Result Correlation Matrix

↓ realised as

Order (Associated Direction Mechanism)

Order Routing Directive (Assertion / Instruction)

Order-Result Correlation Link (Relationship with Provenance & Status)

This retained Foundation example supplies canonical terminology, not Package2 derivation. A ledger/history label does not establish comprehensive retention; correlation does not establish conflict adjudication. Each Function/Service remains in its evidenced owning context without inferring a one-to-one Feature association.

2.4 Bounded Task / Work Responsibility Basis

The Task / Work model traces to Workflow & Activity Coordination's established Functions (`CoordinateWorkOrder`, `CoordinateToDo`, `CoordinateSyntheticTask`, `SuperviseActivityTimeout/Escalation`) and generic Activity Instance Registry, Task State Progression Graph and SLA Deadline & Timer Ledger information responsibilities. Work Order, To Do and Synthetic Task retain their distinct Business meanings and progression authority.

The authorised Task information concepts distinguish archetype, particular work, undertaking and governed ActionableTask resolution without assuming ownership of business meaning or clinical outcome. This responsibility basis allocates no unestablished definition-authoring/outcome-authority owner or specialised archetype/undertaking/outcome subclasses. ActionableTask governs its orthogonal execution-concurrency, outcome-participation and output-rule controls; that information policy does not transfer originating domain authority. ReportedTask and remaining detailed TaskOutcome questions remain unresolved.

3. Scope Boundary for Detailed Model Population

This foundation establishes the canonical derivation mechanism and validates it through representative references.

> **Scope Note**: Full, detailed derivation across the 137 established Strategy Features and their evidenced owning contexts and the complete Domain 03 Business Information Responsibility matrix remains reserved for subsequent, dedicated Domain 04 modelling packages.

52. Cross-Capability Dependencies & Service Exposure Matrix

RAD source: `docs/markdown/03-business-architecture/dependencies/cross-capability-dependencies.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Cross-Capability Dependencies & Service Exposure Matrix

1. Architectural Principles of Cross-Capability Dependencies

In the Harmonia Business Architecture, capabilities do not operate as isolated silos. Healthcare workflows require coordinated collaboration across bounded capabilities.

1.1 Service Exposure and Ownership Boundary Pattern

When Capability A delivers behaviour needed by Capability B, Capability A exposes that behaviour through a formal **Business Service**. Capability B consumes the Service without acquiring ownership of the underlying Function or information:

RAD diagram: textual source notation.

```
graph LR
    subgraph CapA["Capability A (Owner)"]
        FA["Function X"] -->|Exposed as| SA["Business Service X"]
    end
    subgraph CapB["Capability B (Consumer)"]
        CB["Business Behaviour"] -->|Consumes| SA
    end
```

1.2 High-Level Cross-Capability Dependency Topology

The diagram below illustrates the major inter-capability dependency relationships across the five healthcare contextual views:

These nodes are contextual views, not typed Capabilities or identifier ancestors. The arrows summarise established responsibility needs; they do not allocate particular Services, participating Interactions or Collaborations.

RAD diagram: textual source notation.

```
graph TD
    EM["Entity Management<br/>(Identity, Providers, Orgs, Locations, Services)"]
    SA["Service Administration<br/>(Encounters, Referrals, Orders, Documents)"]
    SD["Service Delivery<br/>(Primary, Acute, Emergency, Inpatient, Virtual)"]
    HSO["Health Service Operations<br/>(Wards, Beds, Dispatch, Logistics, Discharge)"]
    ISE["Intrinsic / Shared Enablement<br/>(LHR, HIE, Access, Control, Workflow, Collab)"]

    SA -->|Resolves entities & services| EM
    SD -->|Associates encounters & orders| SA
    SD -->|Retrieves longitudinal record & exchange| ISE
    HSO -->|Coordinates care-places & staff| EM
    HSO -->|Orchestrates operational workflows| ISE
    HSO -->|Synchronises discharge & admissions| SA
    SA -->|Enforces security context & audit| ISE
```

2. Cross-Capability Service Consumption Matrix

The matrix below documents the primary inter-capability service dependencies across Harmonia:

Consuming Capability Context	Exposed Business Service Consumed	Exposing / Owning Capability	Business Purpose
Referral Administration	`PersonIdentifierResolution`	**Person Identity** *(Client Administration)*	Validates source-attributed identifiers against their authoritative issuing information; does not determine a master person.
Referral Administration	`ServiceProvisionResolution`	**Health Service Administration**	Resolves target clinical specialty clinics, receiving providers, and referral catchment rules.
Episode & Encounter Administration	`Care-PlaceSpecificationLookup`	**Location Administration**	Resolves physical ward, room, and bed structural attributes for encounter bed placement.
Episode & Encounter Administration	`BedAvailabilityQuery`	**Bed & Care-Place Management**	Checks real-time operational bed readiness before confirming patient bed moves.
Order Administration	`ServiceProvisionResolution`	**Health Service Administration**	Determines performing diagnostic laboratory or imaging centre routing destinations.
Diagnostic Administration	`TerminologyMappingService`	**Clinical Knowledge Services**	Normalises local laboratory and radiology test codes to canonical SNOMED-CT/LOINC concepts.
Clinical Record Administration	`HealthcareSubjectContextResolution`	**Healthcare Subject**	Validates patient demographic context and indigenous/interpreter requirements for document headers.
Clinical Record Administration	`SemanticConformanceVerificationService`	**Information Design Governance**	Verifies clinical document structural and semantic compliance against governed interchange profiles.
Primary Care	`LongitudinalClinicalRecordQuery`	**Patient Clinical Record**	Retrieves regional patient history, past discharge summaries, and medication timelines for GP consultations.
Acute Care	`ConsentEnforcementService`	**Health Information Control**	Evaluates patient consent directives and break-glass emergency access permissions during acute admissions.

Consuming Capability Context	Exposed Business Service Consumed	Exposing / Owning Capability	Business Purpose
Emergency Care	`PersonIdentifierCorrelation`	**Person Identity** *(Client Administration)*	Resolves established, source-authoritative identity associations for emergency presentations; does not originate probabilistic matching or person-merge decisions.
Inpatient Care	`DischargeReadinessTelemetry`	**Discharge Management**	Coordinates multidisciplinary ward discharge checklists and pharmacy reconciliation.
Bed & Care-Place Management	`Care-PlaceSpecificationLookup`	**Location Administration**	Resolves physical care-place capabilities (e.g., negative pressure, telemetry wiring) for isolation placement.
Work Allocation & Dispatch	`StaffPresenceTelemetryQuery`	**Mobile Staff Management**	Allocates portering and cleaning work orders to active on-duty staff in the nearest physical zone.
Patient Transport	`EncounterLocationHistoryQuery`	**Episode & Encounter Administration**	Validates current patient ward location and target diagnostic department before dispatching porters.
Clinical Logistics Coordination	`SecureClinicalMessageDispatch`	**Clinical Communication Administration**	Sends automated electronic specimen tracking notifications and urgent critical result dispatch alerts.
Workflow & Activity Coordination	`PolicyEvaluation&AuthorisationService`	**Health Information Control**	Evaluates actor task-claiming permissions before allowing work order acceptance or document countersignature.
Presentation Services	`ClinicalInformationSearchService`	**Health Information Access**	Executes search queries across longitudinal clinical records for display in clinical portals.

3. Dependency Governance Rules

The former Clinical Collaboration consumption of `CollaborationClinicalSummaryResolution` from Patient Clinical Record is withdrawn as unsupported under approved G1 K12. Clinical Collaboration retains ownership of that exposed Service through `FEAT-ISE-17` and `ResolveLHRQuerywithinCollaboration`. Its precise source Service/provider, direct or mediated retrieval, formal Service consumer and the incorrect row's intended referent remain unresolved. No replacement dependency is established.

Patient Clinical Record retains longitudinal assembly, active record maintenance, durable preservation and canonical integrated synthesis; Clinical Collaboration retains spaces, membership, discourse, in-conversation resolution and contextual clinical-summary projection; Health

Information Access retains search, retrieval, access-qualified filtering, query execution context and result projection state. Information ownership, Function ownership, Service ownership, consumption, use and presentation remain distinct.

The Order Administration → Practitioner Role Resolution row is withdrawn: the Service resolves Role/capacity information, not Clinical Privilege or Operational Privilege. The Diagnostic Administration → Order Requisition Ingress row is withdrawn: that Service ingests requisitions, not the asserted correlation of incoming reports. Both Services retain their owners and valid exposed behaviour. Required privilege/correlation behaviour remains valid, but precise consuming Service relationships remain unestablished; no replacement edge is inferred.

Clinical Privilege concerns a practitioner's authority/credentials for applicable clinical activity; Operational Privilege concerns an actor's authority for applicable operational activity. Either or both may be required. Practitioner Role information may contribute context but establishes neither.

The former Discharge Management → Clinical Document Lifecycle Service "publication upon departure" assertion is withdrawn. That Service exposes authoring addenda/corrections; the alleged discharge-publication consumption is not established. Discharge Management retains a capability dependency on Clinical Record Administration for applicable discharge information, with the precise Service/provider contract unresolved. Preparation, information availability/publication, discharge authorisation and physical departure remain distinct. Applicable information may be prepared/communicated before departure; `FEAT-HS0-29` dispatch preserves confirmed physical exit as its trigger. No universal publication sequence, additional publication event or replacement Service edge is inferred.

Typed references distinguish Capability dependency, Service consumption, Interaction participation and Collaboration involvement. Abbreviated/descriptive wording is not an alias. Where identity or relationship is not established, retain the reference as unresolved rather than convert it into an edge.

1. **Unidirectional Service Coupling**: Dependency arrows point strictly from the consuming capability to the exposed Business Service. The exposing capability remains completely agnostic of which downstream capabilities consume its services.
2. **Encapsulated Implementation**: Consuming capabilities depend exclusively upon the abstract service contract and its business semantics, never upon internal algorithms or persistence structures.
3. **Fail-Closed Security Context**: All cross-capability service invocations must carry a valid, immutable `SecurityContext` evaluated by *Health Information Control*.

4. Assurance Dependency Derivation Boundary

The approved Strategy contribution matrix establishes contributions to Assurance Design, Assurance Criteria Management and Governed Assurance. It is not a Business Service consumption matrix: neither Direct nor Supporting identifies an exposed Service, its owning Function or an identifiable Business Service consumer. No assurance row or topology edge is added to §2 or §1.2 from those contributions.

Governed Assurance needs applicable criteria and trustworthy evidence. Source-information ownership and subject management remain with their established capabilities. The subject may supply evidence, but that dependency must not give it control of assurance progression or conclusion: **evidence dependency does not compromise assurance independence; control dependency does**. Service Guardian does not acquire assignment, delegation, escalation or remediation responsibility through a finding. Service Assurance Modeller also does not manage the subject merely by designing assurance concerning it.

The approved EC-02 / EC-14 contribution relationship supports **Establish Assurance Context** through generic context-management capability and assurance-specific semantics together. It does not identify an exposed Business Service, a Service consumer, sole realisation, a technical mechanism or an implementation allocation. The five approved Functions establish internal capability-owned behaviour; they do not mechanically create exposed Services or consumption rows.

The approved human review separately establishes the following three Governed Assurance Services and their Role-level consumption/communication relationships. These are approved relationships across the owning responsibility boundary, not inferred from EC contributions or Role-name similarity. No consuming Capability is allocated solely from a Role's name; these assurance relationships do not determine §2's other Capability rows or §1.2's contextual-view topology.

Approved Business Service	Exposing / Owning Capability	Approved consumer / potential recipient Roles	Associated Business Interaction
Request Service Assurance	**Governed Assurance**	Care Coordinator, Service Coordinator, Regulator, Policy Authority, System Steward — authorised requesters	Service Assurance Request
Request Service Assurance Status	**Governed Assurance**	**System Steward only** — stewardship of activity progression	Service Assurance Status
Communicate Service Assurance Outcome	**Governed Assurance**	Care Coordinator, Service Coordinator, Regulator, Policy Authority, System Steward — applicable authorised recipients	Service Assurance Outcome Communication

Requesting does not grant control of assurance assessment/adjudication or authority to alter the applicable Assurance Definition/Criteria. Status concerns assurance activity progression and does not expose unadjudicated findings/conclusions or provide general requester tracking. Outcome communication is not mandatory broadcast; requester, status consumer and recipient need not coincide. Recipients retain their own clinical, operational, governance/compliance and improvement responsibilities; communication transfers none of their subsequent action to Service Guardian.

Assurance Definition use is a responsibility/information dependency between modelling and execution; no Modeller-to-Guardian Interaction or Service is inferred. Source information may be available through existing governed access; evidence association does not create an Evidence Contribution Interaction. Exact evidence/criteria providers, additional exposed behaviour/Services, detailed contracts, consuming Capability allocation, evidence exchange and findings-to-operational-response mechanisms remain unresolved.

Financial Governance is deliberately excluded, while exact authorised external recipients of financially relevant outcomes remain unresolved. Governed Assurance does not recursively assure its own execution; System Steward's status observation is stewardship / management monitoring. Neither boundary establishes an implementation allocation. Existing Service ownership, consumption relationships, security rules and recorded uncertainties remain intact. Role participation alone does not grant information-access authority or bypass governed access.

53. Domain04 Package 2 — G1 Architectural Review Record

RAD source: `docs/markdown/04-information-architecture/reviews/package2-g1-review.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

11. K9 — Capability Metamodel Typing

Status: RESOLVED — APPROVED: CAPABILITY IDENTITIES AND RESPONSIBILITIES PRESERVED; CAPABILITY TIER / ANCESTRY UNRESOLVED

Disposition: Close K9 because the architectural investigation establishes what the current architecture does and does not determine. Preserve established architectural identities, responsibilities, behavioural ownership contexts and catalogued Feature classifications. Retain unresolved Capability Tiers, ancestry, root status and structural Canonical IDs explicitly. Completion of the full Business Enabling Capability hierarchy is not a prerequisite for Domain04 Package 2 derivation. Source reconciliation remains a later, separately authorised activity.

The approved identification prerequisite in §10, the Capability decomposition / derivation model and the Architectural Element Identification metamodel remain controlling. This final disposition supersedes the prerequisite's incomplete-K9 position without changing its identification rules or retrospectively validating legacy classifications. Approved K1–K8, including K5, remain unchanged.

11.1 Approved findings and explicit uncertainty

Architectural dimension	Established finding	Unresolved boundary
Architectural identity	Business Enabling Capability identities are established for the majority of affected elements.	The architectural-element status of every named catalogue decomposition entry is not established.
Semantic responsibility	The relevant responsibilities of those established Capabilities are preserved. Named responsibility contexts for Functions, Services and Processes are substantially established.	Established responsibility does not prove numerical Capability Tier or complete structural ancestry.
Feature type	All explicitly catalogued Features are FT; existing Feature identifiers are preserved. `IdentifierResolution` is FT / `FEAT-EM-01`.	Unsupported Domain03 Feature headings remain governed by approved K5; presentation alone creates no Feature identity.
Capability Tier	CT1 / CT2 / CT3 are the approved decomposition terminology.	No affected legacy L1 / L2 / L3 owner assignment is independently validated sufficiently to establish a CT1 / CT2 / CT3 assignment.
Capability ancestry and root status	Selected explicit parent/responsibility relationships are established, including `Client Administration → Person Identity`.	Complete Capability ancestry and root status for the affected Business Enabling Capabilities remain unresolved. The explicit relationship does not establish either element's tier or Client Administration's root status.

Architectural dimension	Established finding	Unresolved boundary
Structural Canonical ID	The approved identification grammar and established Feature identifiers remain valid.	Complete structural Canonical IDs cannot yet be established where required tier, ancestry or identifier allocation is unresolved.

The Business Enabling Capability catalogue, Domain03 behaviour ownership, Process ownership, Information Responsibility and cross-capability dependencies establish the relevant identities and responsibility contexts within their evidenced boundaries. The explicit `PersonIdentity` parent relationship is stated in Governed Person Identity Correction, §2.1; its disputed L2 label does not establish the tier.

The historical K9 evidence baseline remains evidence of the source assertions requiring reconciliation. Established owner identity, established responsibility and established Feature type SHALL remain distinct from unresolved Capability Tier, ancestry and structural identification. A typing defect SHALL NOT invalidate an otherwise established Capability identity or change its semantic responsibility.

11.2 Legacy classification disposition

No affected legacy L1 / L2 / L3 assignment has been independently validated sufficiently to authorise mechanical translation to CT1 / CT2 / CT3. The investigation categories remain:

Existing assertion	Investigation category	Approved treatment
Affected legacy owner L1 / L2 assignments	C — unsupported / insufficiently evidenced	Preserve the established owning element and responsibility; retain its Capability Tier and complete ancestry as unresolved.
Former `IdentifierResolution` L3 example	D — established Feature incorrectly represented as a Capability Tier	Preserve FT / `FEAT-EM-01`; do not classify it as CT3 or use the former label to establish ancestry.
Former `Closed-LoopOrderProgression` L3 and `DiagnosticOrders` L2 examples	C — unsupported / insufficiently evidenced	The former examples establish no validated Capability Tier or ancestry. Preserve independently evidenced Process responsibility without manufacturing a Capability identity or parent.
Unsupported Domain03 Feature headings	Approved K5 controls	Preserve valid Functions and Services in their evidenced owning context; create no Feature, Feature ID or alias to obtain traceability symmetry.

No affected legacy assignment is established as category A — a semantically validated hierarchy requiring terminology migration alone. Unsupported hierarchy assertions SHALL NOT be treated as mere deprecated wording. `L1 → CT1`, `L2 → CT2` and `L3 → CT3` SHALL NOT be performed as a mechanical migration.

11.3 Downstream derivation rule

> Unresolved Capability Tier or ancestry SHALL NOT prevent downstream architectural derivation where the owning Capability or Feature and its relevant responsibility are otherwise established. Downstream traceability SHALL reference the established architectural element and SHALL NOT manufacture unresolved structural ancestry.

Domain04 Package 2 may therefore derive Information Concepts from established Business Architecture responsibility without waiting for complete CT1 / CT2 / CT3 classification. This permission does not establish missing responsibility or architectural identity where those are themselves unresolved.

Where a full structural Canonical ID cannot yet be established, traceability SHALL use the established architectural type, canonical name and existing identifier where one exists, while

retaining the structural Canonical ID as unresolved. Placeholder structural IDs SHALL NOT be manufactured. An existing Feature identifier SHALL NOT be represented as proof of a complete structural Canonical ID.

11.4 Architectural Element identity and catalogue presentation

> Document structure, catalogue grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity. Canonical Architectural IDs SHALL be assigned only to governed Architectural Elements whose identity and architectural context are established.

Named catalogue decomposition entries SHALL NOT automatically become CT2 or CT3 Capabilities because they appear beneath another Capability. Their architectural-element status may remain unresolved. Historical L1 / L2 / L3 labels and derivation-stage position likewise SHALL NOT serve as self-validating evidence of ancestry or tier.

The five contextual views — Entity Management, Service Administration, Service Delivery, Health Service Operations and Intrinsic / Shared Enablement — remain organisational views. They SHALL NOT automatically become CT1 elements or identifier ancestors. Established Capability contexts beneath a view do not by themselves prove root CT1 status.

11.5 Behavioural ownership and Feature preservation

Named responsibility contexts for affected Domain03 Functions, Services and Processes are substantially established even though their complete structural paths are not. Those behaviours may continue to trace to their established owning Capability or Feature without a complete structural Canonical ID. The identification prerequisite permits direct anchoring at an established Capability Tier or FT context; missing intermediate Capabilities or Features SHALL NOT be manufactured.

All explicitly catalogued Features SHALL remain FT, and all established Feature identifiers SHALL be preserved. The control case is:

Identifier Resolution property	Approved finding
Type	FT
Existing Feature identifier	`FEAT-EM-01`
Owning context	Person Identity
Complete structural Canonical ID	Unresolved

The explicit `Client Administration → Person Identity` relationship and Identifier Resolution's owning context establish only a partial responsibility chain. They SHALL NOT supply missing Capability Tier ancestry or a complete structural Canonical ID.

Approved K5 remains controlling for unsupported Feature headings, existing Feature IDs and canonical naming. Valid Functions and Services do not establish a new Feature merely through a heading or similarity to a broader Feature. No FN / SV / PR identifiers are allocated during K9 persistence.

11.6 Clinical Qualification / Credential distinction

`Clinical Qualification Management` ≠ `Credential Management`.

The approved rename `Clinical Credential Management → Clinical Qualification Management` preserves professional / clinical qualification and competency responsibility. Domain03 `CredentialManagement` retains its separately evidenced operational badge / physical-access responsibility. The elements SHALL NOT be merged or aliased. Neither element's Capability Tier or ancestry is established by the rename or by lexical similarity.

The lexical similarity of Feature or behaviour names SHALL NOT establish semantic identity. In particular, the badge/access-context `OperationalCredentialCheck` behaviour SHALL NOT be

bound to `FEAT-HS0-23` solely by name. That catalogued Feature's qualification/competency responsibility remains distinct from the operational badge/access behaviour.

11.7 Later, separately authorised G1 reconciliation

The minimum later reconciliation SHALL distinguish:

- **Terminology and unsupported hierarchy assertions:** remove or qualify unsupported numerical Capability Tier assertions in the affected sources; use approved terminology only where tier is independently established. Preserve established Capability identities and responsibilities; otherwise retain tier, root status and complete ancestry as unresolved.
- **Evidenced relationship:** preserve `Client Administration → Person Identity` without inventing tiers, intermediate parents or root status. Completion of the hierarchy may occur through later Business Architecture refinement and is not required to close K9.
- **Feature correction:** preserve catalogued FT identities and existing Feature IDs; correct the former Identifier Resolution Capability Tier representation; apply approved K5 to unsupported Feature headings without manufacturing Features or identifiers.
- **Canonical naming correction:** apply approved K5 naming corrections, including Service Delivery naming and Clinical Logistics Coordination, and the approved Clinical Qualification Management rename. Preserve the distinct operational Credential Management element and its responsibility.
- **Downstream traceability correction:** reconcile affected Package 1 traceability labels against established architectural types and owning responsibilities, retaining unresolved structural context explicitly. Preserve the separately approved K3/K6 constraints; do not change Function, Service, Process, Information Responsibility or dependency semantics to repair a type label.
- **Historical evidence and identification:** preserve the evidence baseline, including its historical quotations and fingerprints; allocate no speculative structural Canonical IDs. Catalogue presentation SHALL NOT create governed Architectural Elements or substitute for established context.

Approval of this disposition and authorisation to perform source reconciliation remain separate. This persistence task changes only this durable G1 review record; frozen Domain01–03, Domain04 Foundation and Package 1 sources remain unchanged.

11.8 Closure and stop boundary

2026-10-13 reconciliation: The fixed-decomposition decision changes the target metamodel, not K9's approved evidence, closure or unresolved catalogue status. Complete CT1 → CT2 → CT3 ancestry is now required for a fully reconciled Feature. Existing Features remain valid evidence requiring ancestry reconciliation where their CT3 frame is unestablished. Legacy labels, document structure, contextual views, local Feature IDs and partial ownership SHALL NOT establish missing tiers, complete Canonical IDs or new ancestors. K9's permission to derive from established responsibility remains; it does not declare that responsibility structurally reconciled. The original closure text follows unchanged.

K9 is RESOLVED on the approved interpretation above. Unresolved Capability Tier, ancestry, root status, decomposition-entry identity and structural Canonical ID information remain explicit architectural uncertainties; they do not reopen K9 or require completion of the full Business Enabling Capability hierarchy before downstream derivation from established responsibility.

This disposition preserves AX-01/AX-04 business meaning, AX-14 semantic distinctions and AX-15 uncertainty. It neither changes the frozen Capability Model nor authorises missing identity, ancestry or identifier allocation to be invented. It preserves the approved identification prerequisite and K1–K8 unchanged, completes no G1 reconciliation, derives no Package 2 Information Concept and does not investigate K10 or approve G1 as a whole. Work stops after persistence of this approved K9 disposition and review of the resulting diff.

54. Harmonia Architectural Axioms

RAD source: `docs/markdown/governance/architectural-axioms.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

AX-01 — Harmonia Is Health-Information Centric

Axiom

Harmonia is a distributed health-information management and interoperability framework.

Its primary purpose is the ingestion, validation, transformation, governance, processing, persistence, distribution and exposure of health-related information.

Harmonia SHALL support applicable health-information standards and models while retaining the ability to manage information according to Harmonia's own internal operational requirements.

Rationale

Harmonia is not intended to become a generic distributed application framework to which healthcare happens to be attached.

Its architecture exists to solve health-information problems: interoperability, information authority, provenance, security, terminology, workflow, persistence, distribution, resilience and longitudinal information management.

Consequences

Health-information semantics take precedence over generic infrastructure convenience.

Standards such as HL7 FHIR and HL7 v2 are first-class architectural concerns.

Generic infrastructure capabilities SHOULD be used where useful, but SHALL NOT dictate Harmonia's health-information semantics.

This does not mean

- Harmonia is a FHIR server.
- Harmonia is restricted to FHIR information.
- Every internal Harmonia object must correspond to a healthcare interoperability standard.

AX-04 — Harmonia Owns the Semantics; Engines Provide the Machinery

Axiom

Harmonia owns the architectural semantics governing information authority, security, lifecycle, concurrency, provenance, auditability, resilience and operational integrity.

Harmonia SHALL preferentially use capabilities supplied by underlying technology engines where those capabilities satisfy Harmonia's architectural invariants rather than reproduce equivalent functionality.

Rationale

Harmonia should own its meaning without unnecessarily owning commodity infrastructure machinery.

Consequences

HAPI FHIR MAY provide FHIR persistence, indexing, versioning and search.

Infinispan MAY provide distributed caching, entry metadata and atomic coordination primitives.

ActiveMQ Artemis MAY provide durable messaging.

PostgreSQL MAY provide transactional relational persistence.

These technologies implement Harmonia capabilities. They do not define Harmonia's architectural semantics.

Maxim

Use the machinery. Own the semantics.

This does not mean:

- Harmonia should wrap every engine capability behind a bespoke implementation.
- An engine's native behaviour automatically satisfies a Harmonia invariant.
- Using HAPI FHIR makes Harmonia a HAPI FHIR application.

AX-14 — Semantic Distinctions Are Preserved

Axiom

Harmonia SHALL preserve meaningful distinctions between states, outcomes and concepts throughout its internal processing and across subsystem boundaries.

A distinction that is significant to information meaning, authority, security, concurrency, persistence, processing outcome or operational correctness SHALL NOT be collapsed merely because an underlying technology, transport, API or implementation abstraction does not represent that distinction directly.

Where an external interface contract intentionally presents a simpler or different semantic model, Harmonia MAY project its internal semantics into that contract explicitly at the applicable boundary.

Rationale

Loss of semantic distinction causes information about what Harmonia knows, what occurred and what may safely happen next to be lost.

Two conditions may produce similar implementation behaviour while having materially different meanings. Resource absence, access denial, persistence failure and indeterminate outcome may all prevent information from being returned, but they do not describe the same state.

Likewise, persistence version, active-state generation, interoperability version and information authority describe different properties even where an implementation happens to correlate them.

Implementation convenience SHALL NOT redefine those meanings.

Consequences

Semantically distinct outcomes SHALL remain distinguishable through Harmonia's internal contracts.

Security denial SHALL NOT be represented as resource absence merely because both prevent access to information.

A failure to establish authoritative state SHALL NOT be represented as authoritative absence.

An indeterminate outcome SHALL NOT be represented as either success or failure unless subsequent processing establishes that outcome.

ActiveStateToken, AuthoritativeVersion, externally meaningful version identifiers and standards-defined version metadata SHALL remain distinct where they represent different semantic domains.

Where an underlying engine or protocol lacks a distinction required by Harmonia, Harmonia SHALL introduce an appropriate semantic representation rather than infer meaning from incidental implementation details such as exception classes, message text or correlated version values.

Boundary projection MAY deliberately reduce or transform semantic distinctions where required by an external contract or policy, but such projection SHALL be explicit and SHALL NOT redefine Harmonia's internal semantics.

Maxim

Preserve semantics internally; project deliberately at boundaries.

This does not mean: Harmonia must expose every internal distinction externally. Similar outcomes can never share implementation machinery. Every semantic distinction requires a separate Java type. External standards or interface contracts may be ignored in favour of Harmonia's internal model.

AX-17 — Architectural Authority and Explicit Uncertainty

Axiom

Documented architectural decisions, definitions, relationships, boundaries and constraints are authoritative and SHALL NOT be silently reinterpreted, replaced, bypassed or contradicted by downstream architecture or implementation.

Where the architecture does not establish a fact, relationship, responsibility or decision, that absence SHALL be preserved explicitly rather than completed through inference, convention, structural convenience, lexical similarity or anticipated implementation.

Unresolved architecture MAY be accompanied by clearly identified candidates, suggestions or recommendations, but these SHALL remain explicitly non-authoritative until explicitly accepted into the architecture.

Where evidence indicates that authoritative architecture may be incorrect, incomplete or internally inconsistent, the conflict SHALL be raised explicitly for architectural review. The existing architecture remains authoritative until an approved architectural change is made. That approved change becomes the new authoritative architecture.

Rationale

Downstream derivation must distinguish established architectural knowledge from gaps and proposed resolutions. A plausible relationship supplies no authority merely because it makes a model or implementation appear complete. Absence of an established relationship is architecturally meaningful information.

Repeated architectural review during Domain04 Package2 G1 confirmed this need across different traceability relationships:

- K9: unresolved Capability hierarchy must not be completed through invented ancestry.
- K10: Behaviour / Process traceability must not be completed through invented stage equivalence.
- K11: Function / Service traceability must not be completed through unsupported Feature association.
- K12: cross-capability dependency traceability must not be completed through invented Service ownership or consumption.
- K13: downstream representation traceability must not be completed through anticipated FHIR, application or technology mappings.

These examples justify an architecture-wide rule; its application does not depend on Domain04 or G1 terminology.

Consequences

Human architects, human developers and AI development agents SHALL apply this axiom to architecture derivation, documentation generation and implementation.

Architectural traceability SHALL represent established relationships and explicitly preserve unresolved or unestablished relationships. Completeness SHALL NOT be manufactured through:

- inferred hierarchy;
- structural convenience;
- lexical similarity;
- conventional modelling patterns;
- assumed equivalence;
- anticipated implementation;
- familiar standards;
- probable application design.

Candidates, suggestions and recommendations SHALL be distinguishable from documented, approved architecture. A proposed resolution or apparently better alternative SHALL NOT acquire authority through downstream use.

AI development agents SHOULD challenge architecture when evidence warrants architectural review. They SHALL NOT silently bypass it because they recognise a familiar destination.

This axiom applies the authority and explicit-review obligations in Purpose, Relationship to Architecture Decisions and Architectural Review Rule to downstream derivation and implementation.

AX-14 preserves semantic distinctions; this axiom preserves the distinction between established, unresolved and proposed architecture. AX-15 governs uncertainty about operational state, outcome or effect; this axiom governs uncertainty about the architecture itself. Neither form of uncertainty may be silently resolved by assumption.

AX-02 and AX-04 preserve Harmonia's internal semantics from external standards and engine assumptions. Familiar standards or technology patterns do not establish otherwise undocumented architectural relationships. AX-06 governs the authority of managed information; architectural authority remains a distinct concern.

Maxim

Traceability must be truthful, not artificially complete.

AX-18 — Integration and Coordination Do Not Confer Domain Authority

Axiom

> Harmonia MAY integrate information concerning, and coordinate operational activity associated with, an externally governed domain activity without thereby acquiring authority to determine or manage that domain activity.
>
> Authority to determine or manage a domain activity SHALL remain with the responsible actor, system or governed capability unless that authority is explicitly assigned to Harmonia by the architecture.
>
> Harmonia's observation, processing, persistence, distribution, presentation or use of information concerning a domain activity SHALL NOT, by itself, establish authority over that activity.

Rationale

Participation in a domain activity and authority over that activity are different architectural concerns. Information possession, observation, integration, coordination and processing do not, by themselves, confer domain authority. Authority requires explicit architectural assignment.

This new human-approved axiom generalises an already-established class of architectural boundary. It is established at the 2026-10-09 migration boundary; it is not a claim that AX-01 through AX-17 always contained this axiom.

Consequences and qualifications

AX-18 SHALL NOT be interpreted as preventing Harmonia from performing operational activity for which architecture has assigned Harmonia responsibility. Explicit architectural assignment may establish authority to determine or manage a domain activity.

In particular, AX-18 does not mean:

- Harmonia cannot execute Praxis/Ergo activity;
- Harmonia cannot coordinate entity-specific operational activity through a Digital Twin;
- Harmonia cannot process or transform clinical information;
- Harmonia cannot perform workflow/activity coordination;
- Harmonia cannot manage responsibilities explicitly assigned to Harmonia;
- Harmonia cannot enforce policy, security, information governance or operational controls assigned to it.

Participation, information, integration and coordination do not, by themselves, establish authority to determine or manage the underlying domain activity. Assigned Harmonia operational responsibility remains applicable.

Examples and established specialisations (non-normative)

These examples explain the boundary; they do not expand the axiom or establish Application Components, Information Concepts, Services, Processes, AI allocation or implementation mechanisms.

- **Clinical work:** Harmonia may integrate and coordinate around clinical work without thereby acquiring authority to determine clinical work, manage clinical handover, own clinical worklists, allocate clinical work, make clinical decisions or become the EMR workflow authority. The detailed Domain03 clinical-work integration boundary remains the Business specialisation.
- **Digital Twins:** A Twin may coordinate operational activity associated with its real-world entity without thereby acquiring authority over originating clinical facts or domain decisions. The Strategy entity-specific coordination rule remains controlling for that coordination.

- **Capacity:** Higher-order Service Capacity Management may consume context-specific capacity without acquiring the originating responsibility for Clinic, Ward, Theatre, ED, Outpatient or Mobile Service Capacity Management. The detailed Business capacity responsibility boundary remains in Business Architecture.
- **Assurance:** Observation, access to evidence or establishment of an assurance conclusion does not confer management authority over the activity being assured. Independent Assurance remains governed by REQ-FND-005 and its established Strategy and Business derivation.
- **Presentation / UI:** Presentation or collection of information does not, by itself, confer originating information or domain-decision authority on the presenting component. The existing Iris contextual human interaction responsibility remains distinct.
- **AI:** Assistance with interpretation, pattern recognition, criteria evaluation or decision support does not, by itself, confer the authority of the governed actor, role or capability using that assistance. This example assigns no AI responsibility or capability.

Related architectural authority

- AX-06 governs information authority and credibility.
- AX-13 governs the Harmonia management boundary at ingress/egress.
- AX-14 preserves meaningful semantic distinctions.
- AX-16 governs coordinated operational activity/entity progression.
- AX-17 governs architectural authority and explicit uncertainty.
- AX-18 governs inference of domain authority from participation, information, integration or coordination.

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